MSc Marriage and Family Therapy

The Lecture Library

Twenty-eight course books in one file

This single file contains every volume of the library, in programme order: eight courses in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. Each course keeps its own cover, imprint, contents, running heads and folio, so the file can be printed whole or the individual volumes printed from the separate PDFs in `pdf/`.

Use the bookmark panel of your PDF reader to jump to any course or to any chapter within a course. Every volume is also available as a standalone file, and the same text is published as a browsable site in `site/`.

Courses in this library

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents of each volume: how to use the book; course description and rationale; learning outcomes; assessment; the fourteen-week plan; fourteen teaching weeks; an integration chapter; a glossary; a self-assessment paper with model answers; and further reading.

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 1 OF 28

MFT 701

Fundamentals of Marriage and Family Therapy

14 teaching weeks 32 practicals 19 worked cases 1 figure

SEMESTER ONE

1.71 units Prerequisite: none

> This is the course that turns a helping professional into a systemic one.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 701 Fundamentals of Marriage and Family Therapy

MSc Marriage and Family Therapy Semester One 1.71 Units 14 teaching weeks Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 701 Fundamentals of Marriage and Family Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	**Fundamentals of Marriage and Family Therapy**	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 15

1. Course description and rationale . . . 15

2. Learning outcomes . . . 16

3. Assessment . . . 16

4. The fourteen-week teaching plan . . . 16

Week 1 What is family therapy? The relational turn . . . 17

1.1 The lecture in ninety minutes . . . 17

1.2 Why the field exists: three failures of the individual frame . . . 18

1.3 What "relational" actually means: five claims . . . 19

1.4 The four consequences, worked through . . . 21

1.5 Systems as a way of thinking . . . 22

1.11a What the evidence says, in one page . . . 23

1.6 First contact: three openings, annotated . . . 24

1.7 The Okonkwo case, fully worked . . . 26

1.8 Where the profession sits . . . 27

1.9 Culture, faith and class in the first meeting . . . 28

1.10 Six errors that begin in week one . . . 29

1.11 Laboratory: the widening question . . . 30

1.12 Reflective task . . . 31

1.13 Self-test . . . 31

1.14 Readings for this week . . . 31

1.6a An extended transcript, annotated . . . 32

1.10a Three punctuations of one quarrel . . . 33

1.13a Examination question with model answer . . . 34

1.13b What "good" looks like at the end of week one . . . 35

Week 2 Linear and circular causality . . . 35

2.1 The lecture in ninety minutes . . . 35

2.2 What a cause is: four traditions you will use without knowing . . . 36

2.3 Feedback, formally . . . 37

2.4 Three ways of drawing the same family . . . 38

2.4a A worked loop, with numbers . . . 39

2.5 Punctuation, worked . . . 40
2.6 Circular questioning . . . 41
2.6a Reframing: the intervention that follows from circularity . . . 42
2.7 Hypotheses: how to hold one without being owned by it . . . 44
2.8 The Adeyemi family, worked: from loop to plan . . . 44
2.9 Annotated transcript: turning a linear account into a loop . . . 46
2.10 Laboratory: the same case, three drawings . . . 47
2.11 Six common errors with circularity . . . 48
2.12 Culture, faith and the limits of circularity . . . 49
2.13 Key terms for this week . . . 49
2.14 Evidence note: does circularity survive contact with the evidence? . . . 50
2.15 Self-test . . . 51
2.16 Reflective task . . . 52
2.17 Readings for this week . . . 52
Week 3 A short history of the field . . . 53
3.1 The lecture in ninety minutes . . . 53
3.2 Prehistory: three streams before "family therapy" . . . 54
3.3 Wave 1: communication (1952 1970) . . . 54
3.4 Wave 2: structure and generations (1965 1980) . . . 55
3.5 Wave 3: meaning (1975 1990) . . . 56
3.6 Wave 4: solutions and stories (1980 2000) . . . 57
3.7 Wave 5: evidence, attachment and integration (1990 present) . . . 57
3.8 The family tree as a diagram . . . 58
3.9 What each wave still gives you . . . 59
3.10 The Adeyemi family seen through five lenses . . . 60
3.10a Four primary sources, read closely . . . 61
3.11 Annotated transcript: history in one sentence . . . 62
3.11a How the field's arguments became clinical habits . . . 63
3.12 Six errors in using history . . . 64
3.12a A supervision case: what happens when the model fails . . . 64
3.13 Culture, faith and the Nigerian lineage . . . 65
3.13a The Nigerian and African lineage, in more detail . . . 65
3.14 The comparative essay: how to write it . . . 66
3.15 Timeline for revision . . . 67

3.16 Self-test . . . 68
3.16a What the examiners look for . . . 69
3.17 Reflective task . . . 70
3.18 Readings for this week . . . 70
3.16b Two worked formulations, same family, different waves . . . 70
Week 4 Systems concepts you will actually use . . . 71
4.1 The lecture in ninety minutes . . . 71
4.2 What makes a concept usable . . . 72
4.3 Boundaries . . . 72
4.4 Subsystems and hierarchy . . . 73
4.5 Triangles, coalitions, detouring: three different things . . . 74
4.5a A triangle worked, carefully . . . 75
4.6 Rules, roles, myths and secrets . . . 76
4.7 Communication concepts worth keeping . . . 77
4.8 The genogram as a conceptual instrument . . . 77
4.9 The Adeyemi family, mapped . . . 78
4.10 Annotated transcript: mapping in the room . . . 79
4.10a Second transcript: naming the rule, not the person . . . 80
4.11 Laboratory: the structural map . . . 80
4.12 Six errors in using systems concepts . . . 81
4.12a Writing a structural formulation for a report . . . 81
4.13 Culture, faith and the boundary question . . . 82
4.13b A five-minute version for the corridor . . . 83
4.13a When the concept is the problem: the misuse audit . . . 83
4.14 The concept table you will actually use . . . 84
4.15 Two families, same concept, opposite interventions . . . 85
4.16 Evidence note: which of these concepts have research support . . . 86
4.17 Key terms . . . 87
4.18 Self-test . . . 87
4.19 Reflective task . . . 88
4.21 Appendix: the case bank for this week . . . 88
4.20 Readings for this week . . . 89
Week 5 The profession: identity, scope and settings . . . 89

5.1 The lecture in ninety minutes . . . 89
5.2 What makes an occupation a profession . . . 90
5.3 Scope of practice: three circles, not one . . . 91
5.4 Training, titles and what they may claim . . . 92
5.5 Six settings and their invisible contracts . . . 93
5.6 A day in three settings . . . 94
5.7 Working in a multidisciplinary team . . . 95
5.8 Referral: the skill nobody teaches . . . 96
5.9 Money, contracts and the ordinary ethics of practice . . . 97
5.10 The law, at the level a therapist needs . . . 98
5.11 The long career: supervision, CPD and burnout . . . 98
5.12 Annotated transcript: the contract conversation . . . 99
5.13 Laboratory: the scope-of-practice tribunal . . . 100
5.14 Six errors in professional identity . . . 101
5.15 Culture, faith and the professional's own position . . . 102
5.16 Evidence note: does professionalisation improve outcomes? . . . 102
5.17 Key terms . . . 103
5.18 Self-test . . . 103
5.19 Readings for this week . . . 104
5.5a Two more settings: private practice and telehealth . . . 105
5.7a The network as a system: case management when several services are involved . . . 105
5.11a Writing your first professional documents . . . 106
5.12a The professional's first year: a survival map . . . 107
Week 6 The first session I: joining . . . 107
6.1 The lecture in ninety minutes . . . 107
6.2 What joining is . . . 108
6.3 Why the first session is an outcome event . . . 108
6.4 Preparing the session . . . 109
6.5 The opening ten minutes: an architecture . . . 110
6.5a Reading a family's style in seven minutes . . . 111
6.6 Joining each member . . . 112
6.7 Handling four hard families . . . 113
6.8 The Adeyemi first session, in full . . . 113
6.8a The Okonkwo first session, contrasted . . . 114

6.9 Annotated transcript: joining the father who did not come . . . 115
6.10 Laboratory: the joining gate . . . 116
6.10a The feedback sequence, scripted . . . 117
6.10b Two opening scripts you can memorise . . . 118
6.11 Six errors in the first session . . . 119
6.12 Culture, faith and the first ten minutes . . . 120
6.12a The joined room: what good looks like from outside . . . 121
6.13 Evidence note: what predicts a second session . . . 122
6.14 Key terms . . . 122
6.15 Self-test . . . 123
6.16 Reflective task . . . 123
6.17 Readings for this week . . . 124
Week 7 The first session II: defining the problem . . . 124
7.1 The lecture in ninety minutes . . . 124
7.2 Why the definition is the intervention . . . 125
7.3 The four criteria, tested . . . 126
7.4 From complaint to request: the negotiation . . . 126
7.5 Reframing the definition . . . 127
7.6 Presenting problem, underlying problem, and what to leave alone . . . 128
7.7 The agreed statement . . . 129
7.7a The Adeyemi definition, negotiated in real time . . . 129
7.8 Annotated transcript: negotiating with a family that does not agree . . . 131
7.9 Laboratory: the definition negotiation . . . 132
7.9a The goals grid: four columns, one page . . . 133
7.10 Six errors in defining the problem . . . 133
7.10a Rewriting six institutional definitions . . . 134
7.10b The definition audit: five questions after every first session . . . 135
7.11 Culture, faith and the definition of a problem . . . 135
7.11a Working with a family whose definition is a diagnosis . . . 136
7.12 Evidence note: goal consensus and outcome . . . 137
7.12a Two definitions, two plans: the same family, a year apart . . . 137
7.13 Key terms . . . 138
7.14 Self-test . . . 138
7.15 Reflective task . . . 139

7.16 Readings for this week . . . 139
7.10b The definition audit: five questions after every first session . . . 140
7.11a Working with a family whose definition is a diagnosis . . . 140
7.12a Two definitions, two plans: the same family, a year apart . . . 141
Week 8 The genogram as a hypothesis machine . . . 142
8.1 The lecture in ninety minutes . . . 142
8.2 What a genogram is for . . . 142
8.2a What the map cannot show . . . 143
8.3 Symbols, conventions and the discipline of drawing . . . 143
8.4 The interview: a standard sequence . . . 144
8.4a When a member will not take part . . . 145
8.5 Reading the map: six patterns that recur . . . 146
8.6 The date-line: the most powerful question in the technique . . . 147
8.6a Three date-line cases from the case bank . . . 147
8.7 Drawing with the family present . . . 148
8.7a Mapping three household structures . . . 149
8.8 Annotated transcript: the genogram that changed the session . . . 150
8.9 Laboratory: the three-generation genogram . . . 152
8.9a Reading a published genogram: an exercise in hypothesis generation . . . 153
8.10 Six errors in genogram work . . . 154
8.10a The ten-minute genogram . . . 154
8.11 Culture, faith and the genogram . . . 154
8.12 Evidence note: what the genogram is and is not . . . 155
8.12a Putting the genogram in the record . . . 156
8.12b Two genograms of one family, and the difference it made . . . 156
8.13 Key terms . . . 157
8.14 Self-test . . . 158
8.15 Reflective task . . . 158
8.16 Readings for this week . . . 159
Week 9 Formulating relationally . . . 159
9.1 The lecture in ninety minutes . . . 159
9.2 What a formulation is, and the five jobs it does . . . 160
9.2a Bringing the numbers into the formulation . . . 161

9.3 The 4 Ps in a systemic key . . . 161
9.4 A one-page relational formulation template . . . 163
9.5 Worked formulation: the Adeyemi family, six sessions in . . . 163
9.5a The network map: one case, drawn . . . 165
9.6 Four lenses on one family . . . 166
9.7 Annotated transcript: presenting a formulation to a family . . . 167
9.8 Laboratory: three formulations, one transcript . . . 169
9.9 Six errors in formulation . . . 169
9.10 Culture, faith and power in the formulation . . . 170
9.10a Formulating across language, and with an interpreter . . . 171
9.11 Evidence note: how strong is the evidence for formulation? . . . 171
9.12 The multidisciplinary network: formulating the system around the family . . . 172
9.13 Writing it: the record, the report and the letter . . . 174
9.14 Key terms . . . 175
9.15 Self-test . . . 175
9.16 Reflective task . . . 176
9.17 Readings for this week . . . 176
Week 10 Culture, class and power . . . 176
10.1 The lecture in ninety minutes . . . 176
10.2 Three mistakes . . . 177
10.3 The cultural genogram . . . 178
10.4 Class, money and material conditions . . . 179
10.5 Power in the room . . . 180
10.5a Structural competency: when the problem is the institution . . . 181
10.6 Worked case: when duty and desire collide . . . 181
10.7 Annotated transcript: naming a cultural assumption in the room . . . 183
10.7a Working with a community or religious co-worker . . . 184
10.8 Faith as the organising frame . . . 184
10.9 Discrimination, tribe and the family's experience of institutions . . . 185
10.10 Language, naming and address . . . 186
10.11 Gender, hierarchy and the limits of cultural respect . . . 187
10.11a Worked decision: a discipline case at the threshold . . . 188
10.12 Migration and the acculturation gap . . . 189
10.12a Two migration cases, worked briefly . . . 190

10.13 Evidence note: does cultural adaptation improve outcomes? . . . 190
10.14 Six errors . . . 191
10.15 Key terms . . . 191
10.16 Self-test . . . 192
10.17 Reflective task . . . 193
10.18 Readings for this week . . . 193
Week 11 What the evidence says . . . 193
11.1 The lecture in ninety minutes . . . 193
11.2 Why a clinician needs research literacy . . . 194
11.2a How evidence gets into a service . . . 194
11.3 Designs, and the questions they can answer . . . 195
11.4 Effect sizes in plain language . . . 196
11.4a Reading a forest plot . . . 197
11.5 The map of the evidence . . . 197
11.5a Two studies that changed practice, described properly . . . 199
11.6 Reading a paper in twenty minutes . . . 200
11.6a Where to find the evidence quickly . . . 201
11.7 Annotated abstract: reading between the lines . . . 201
11.7a Appraising a qualitative study . . . 202
11.8 Routine outcome monitoring and feedback . . . 203
11.8a Choosing measures for a family clinic . . . 203
11.9 Common factors, specific factors, and the therapist . . . 204
11.9a The replication problem, and what it means for a trainee . . . 205
11.9b Two measures of the alliance, and how to use them . . . 205
11.10 The family's own evidence . . . 206
11.10a The one-page evidence sheet . . . 206
11.11 Six errors in using evidence . . . 207
11.11a The journal club that works . . . 207
11.12 Culture, faith and the evidence . . . 208
11.13 Evidence note: where the field is genuinely contested . . . 209
11.14 Key terms . . . 209
11.15 Self-test . . . 210
11.16 Reflective task . . . 211
11.17 Readings for this week . . . 211

Week 12 Ethics, risk and the limits of the systemic frame 211
12.1 The lecture in ninety minutes 211
12.2 Four ways the systemic frame can be misused 212
12.2a Four principles, applied to a family 212
12.3 Confidentiality in a multiperson system 213
12.4 Risk: the screen, the assessment, the plan 214
12.4b Domestic abuse: a screen that changes the work 215
12.5 Competence, and the duty to refer 216
12.6 Dual relationships and power in small communities 217
12.7 Documentation, and the record as an ethical object 217
12.7b Who owns the record? 218
12.8 The limits of the systemic frame 218
12.9 Six ethical errors 219
12.9a Case: the secret that involves an institution 219
12.10 Key terms 220
12.11 Self-test 221
12.12 Reflective task 222
12.13 Readings for this week 222
12.3a Annotated transcript: setting the frame in ninety seconds 222
12.4a Suicide risk in a family session 224
12.5a Mandated families: the ethics of court-referred work 225
12.5b Three worked competence decisions 225
12.6a Five dual-relationship scenarios, decided 226
12.7a A note that survives scrutiny 227
12.8a Referring well: the handover 228
12.10a When a complaint is made 228
Week 13 Integration I: three lenses on one family 229
13.1 The lecture in ninety minutes 229
13.2 Three kinds of integration 229
13.2a Your primary model: a diagnostic 230
13.3 The case: the Adeyemis, session one 231
13.4 The structural lens 231
13.5 The Bowen lens 232

13.5a The same case at session eight: what each lens would call progress . . . 233
13.6 The solution-focused lens . . . 234
13.7 Two further lenses, briefly . . . 235
13.8 The three plans side by side . . . 235
13.8a When the lenses disagree: a decision protocol . . . 236
13.9 Annotated transcript: switching lens mid-session . . . 237
13.9a Rehearsing the switch: three scripts . . . 238
13.10 Laboratory: three plans, one decision . . . 239
13.10a A fourth lens: the institution's view of the same case . . . 240
13.11 Six errors of integration . . . 240
13.11a A second data set: Folake and Dele . . . 241
13.12 Culture, faith and the choice of lens . . . 242
13.13 Evidence note: does integration work? . . . 243
13.13a Two transcripts of the same moment, in two frames . . . 243
13.14 Key terms . . . 244
13.15 Self-test . . . 244
13.16 Reflective task . . . 245
13.17 Readings for this week . . . 245
Week 14 Integration II: your emerging clinical stance . . . 246
14.1 The lecture in ninety minutes . . . 246
14.2 What a stance is . . . 246
14.2a Where a stance comes from . . . 247
14.3 The theoretical commitment . . . 248
14.4 The epistemological commitment . . . 248
14.5 The ethical commitment . . . 249
14.5a A supervision transcript: a stance under pressure . . . 249
14.6 The cultural commitment . . . 250
14.7 The acknowledged limit . . . 251
14.8 Writing the statement: structure and a worked model . . . 252
14.9 Two statements compared . . . 253
14.9a The statement as a working document . . . 254
14.10 The viva: defending a stance under challenge . . . 255
14.10a A worked viva, in full . . . 256
14.11 The development plan: the first six months . . . 256

14.11a Where this course leads . . . 257
14.12 Ending well: how the course, the placement and the therapy end . . . 258
14.12a The last session with a family, annotated . . . 258
14.13 Six errors in developing a stance . . . 259
14.14 Key terms . . . 260
14.14a The three-year review: keeping a stance alive . . . 260
14.14b Two endings, and what each teaches . . . 261
14.15 Self-test . . . 261
14.16 Reflective task . . . 262
14.17 Readings for this week . . . 262
Integration: the whole course on two pages . . . 263
Glossary . . . 264
Self-assessment . . . 264
Multiple choice (answers below) . . . 264
Answer key . . . 265
Short answer (150 words each) . . . 265
Essay questions (choose one, 1,500 words) . . . 265
Practical and clinical log requirements . . . 265
Further reading . . . 266

Course leader's summary. This is the course that turns a helping professional into a systemic one. It covers the intellectual history of family therapy, the epistemology that separates it from individual work, the profession's identity and scope, and the basic architecture of a first session. Everything later in the programme — theory, technique, assessment, ethics — assumes the vocabulary built here.

How to use this book

Each week contains four things: **the idea** (what you must understand), **the practice** (what you must be able to do), **the case** (what it looks like in a room), and **a reflective task** (what it asks of you personally). Read the week before the lecture; do the practical in the lab; write the reflection within 48 hours, while it is still uncomfortable.

The book is written to be read straight through, but it is also designed for revision: the integration chapter at the end compresses the whole course into two pages, and the glossary defines every term the examination may use.

1. Course description and rationale

Most mental health training teaches us to look inside a person: their thoughts, their symptoms, their attachment history. Family therapy teaches us to look between people: the loop, the triangle, the rule that nobody wrote down. This is not a rejection of individual psychology. It is an additional discipline of attention, and it changes what counts as the problem, the cause, and the cure.

Consider three common presentations:

Presentation	Individual formulation	Systemic formulation
A 15-year-old refuses school	Anxiety disorder, school phobia	A child whose symptoms hold a parental conflict at bay; the school refusal is the family's most effective way of preventing an argument about the marriage
A wife is "depressed"	Major depressive episode	A woman in a triad with her husband and his mother, carrying the family's unspoken grief about a lost pregnancy
A man "works too much"	Workaholism, avoidance	The person who regulates the family's emotional temperature by being absent, in a system that rewards his absence

None of these systemic formulations is automatically more true. Each is a hypothesis with different implications for where you intervene. The mature clinician holds both — the individual and the relational — and chooses deliberately.

Why this course first. Because students who begin with technique learn to do something they cannot explain, and students who begin with theory learn to explain something they cannot do. MFT 701 begins with framing: what kind of problem is this, and therefore what kind of help is required?

2. Learning outcomes

By the end of the course, students will be able to:

#	Outcome	Assessed in
LO1	Explain the epistemological shift from linear to circular causality, and its clinical consequences	Exam, Week 2 log
LO2	Trace the historical development of family therapy and locate contemporary models within it	Exam
LO3	Describe the profession's identity, scope of practice, and relationship to allied professions	Assignment
LO4	Conduct a structured first session, including joining, problem definition, and contracting	Skills assessment
LO5	Draw a three-generation genogram and generate hypotheses from it	Assignment
LO6	Critically discuss the common-factors evidence and its implications for model choice	Exam essay
LO7	Identify their own family-of-origin patterns and describe their effect on clinical work	Reflective log

3. Assessment

Component	Weight	Due
Reflective practice log (weekly, 300 400 words)	20%	Fortnightly
Genogram and hypothesis assignment (2,000 words + diagram)	30%	Week 9
Two-hour written examination (seen case + essay + short answers)	50%	Week 15

Skills gate. Students must pass a 15-minute observed first session (Week 6) before proceeding to MFT 706. The gate is pass/fail with one resit.

4. The fourteen-week teaching plan

Week	Topic	Practical	Core reading
1	What is family therapy? The relational turn	Triad: "the problem as described"	Nichols ch. 1; Handbook ch. 1
2	Linear and circular causality; first- and second-order cybernetics	Observation of a recorded argument	Watzlawick, Pragmatics ch. 1
3	A short history: from schizophrenia research to evidence-based practice	Timeline construction	Nichols ch. 2 3
4	Systems thinking: boundaries, feedback, homeostasis, rules	Feedback-loop mapping exercise	Becvar & Becvar ch. 2

Week	Topic	Practical	Core reading
5	The profession: identity, scope, settings, teamwork	Role boundaries case discussion	AAMFT code, scope sections
6	The first session I: joining, and the referral question	**Observed skills gate**	Minuchin & Fishman ch. 1
7	The first session II: defining the problem in the family's language	Triad: problem definition	Haley, Leaving Home ch. 1
8	The genogram as a hypothesis machine	Genogram lab (own family)	McGoldrick, Genograms ch. 1 2
9	Formulating relationally: the 5-P grid applied to a family	Formulation workshop	Dallos & Draper ch. 4
10	Culture, class and power in the consulting room	Reflexivity exercise	McGoldrick, Ethnicity & Family Therapy
11	Evidence: what works, for whom, and the common factors	Appraisal of one RCT and one qualitative study	Wampold ch. 2
12	Ethics, risk and the limits of the systemic frame	Risk-screen rehearsal	MFT 704 pre-reading
13	Integration I: three models applied to one family	Live demonstration + discussion	Nichols ch. 10
14	Integration II: your emerging clinical stance	Position statement presentation	Course review

Week 1 What is family therapy? The relational turn

1.1 The lecture in ninety minutes

Week 1 has one job: to make the relational frame feel inevitable rather than eccentric. Everything a student does later the first session, the genogram, the formulation, the ethics of conjoint work assumes they can see a relationship as a thing with a shape, a history and a set of rules. Students arrive able to see people. Very few arrive able to see between people.

The ninety minutes run like this.

Minutes	Segment	What the lecturer does	What students do
0 10	Two formulations	Read the Okonkwo referral twice: once as an individual case, once as a relational one	Note which version they would have written
10 30	Where the field came from	The three failures of the individual frame (1.2)	Timeline exercise: place five founding texts
30 50	Five claims of the relational frame	Build the list on the board, testing each against a live example	Vote: which claim is hardest to accept?
50 65	Systems as a way of thinking	Cybernetics, feedback, the Watzlawick axioms (1.5)	Map one family loop on paper

Minutes	Segment	What the lecturer does	What students do
65 80	Three openings in a first session	Annotated transcript, read aloud in parts	Mark where the therapist widened the field
80 90	Set the lab and the journal	Hand out the widening-question protocol (1.11)	Form triads, choose a real (non-clinical) difficulty

The lecture is deliberately front-loaded with history. Students who know the field's origins stop treating systemic thinking as a technique among techniques and start treating it as an epistemology a claim about what kind of thing a problem is.

1.2 Why the field exists: three failures of the individual frame

Family therapy was not invented because someone preferred groups to individuals. It was invented because three well-funded, well-intentioned research programmes ran into the same wall: the individual frame kept losing information that mattered clinically.

The first failure: the patient kept relapsing into a family. In the 1950s, Gregory Bateson's research group at Palo Alto Bateson, Don Jackson, Jay Haley and John Weakland was studying communication in families where one member had been diagnosed with schizophrenia. Their 1956 paper, "Toward a theory of schizophrenia," proposed the **double bind**: a pattern in which a person receives two messages at different logical levels that contradict each other, cannot comment on the contradiction, and cannot leave the field. A mother who says "come here, dear" while stiffening when the child approaches; a father who asks for honesty and then punishes it; a family that requires affection and forbids the expression of need that would make affection possible.

The double-bind theory has not survived as an explanation of schizophrenia. Its status as a causal claim about psychosis is dead, and students should know that. But something else in that research survived and became the spine of the field: the observation that **sequences of behaviour between people carry information that is invisible when you describe the people separately.** The patient's behaviour was intelligible as a move in a game; the game was not in the patient.

The second failure: the symptom made sense at home. Independently, researchers studying families of people with serious mental illness Theodore Lidz at Yale, Lyman Wynne at the National Institute of Mental Health noticed that families organised themselves around the symptomatic member. Wynne described **pseudo-mutuality**: a family that maintains the appearance of harmony by prohibiting any relationship that would threaten it. Lidz described **marital schism** and **marital skew**: two patterns in which the couple's conflict or the couple's imbalance shaped the child's development. Again the aetiological claims are dated. Again the structural insight held: **a family can be organised by a symptom**, and the symptom can be doing work for the system.

The third failure: the child was the family's messenger. The child guidance movement of the 1920s 1950s treated the child, advised the mother, and sent both home to a household that had not changed. Clinicians began to notice that improvement in the child was routinely followed by a new symptom in a sibling, or in the mother, or by a marital crisis that had been dormant. The pattern is now so familiar that it has a name

symptom substitution and the observation that produced it is the most practically useful idea in this week: if the symptom is functional for the system, removing the symptom without changing the system is unstable.

Three research programmes, three different populations, one conclusion: the unit of assessment was wrong. Between 1950 and 1975 the field built an alternative. Nathan Ackerman founded the Family Institute in 1960 and, with Jackson, the journal Family Process in 1962. Murray Bowen developed his theory of differentiation and multigenerational transmission at the Menninger Clinic and later at Georgetown. Virginia Satir was teaching conjoint family interviewing in California. Salvador Minuchin, working with delinquent boys at the Wiltwyck School and then at the Philadelphia Child Guidance Clinic, built structural family therapy out of the observation that these families were not sick so much as **organised at the wrong level of hierarchy** children in charge, parents disengaged or in coalition, boundaries too diffuse or too rigid. Jay Haley joined Minuchin in Philadelphia in 1967 and brought strategy, power and the deliberate use of directives.

By the late 1970s the field had a canon: the Palo Alto communication school, Bowen's multigenerational theory, Minuchin's structural therapy, Haley and Madanes's strategic therapy, the Milan group's systemic therapy with its rituals and positive connotation, Satir's human validation, and the beginnings of solution-focused work at Milwaukee and narrative therapy in Adelaide and Auckland. Students will meet each of these properly in MFT 702. For now, one thing matters: **every one of these schools is an answer to the question "what is the unit?"** and each answers it slightly differently the sequence, the generations, the structure, the meaning, the solution, the story.

> **DEBATE**
> "If the aetiological claims of the 1950s were wrong, the field built on them is unsound." Ten minutes, two sides. The strongest argument against: a science is allowed to keep its methods after discarding its first theories medicine kept auscultation after humoral theory fell. The strongest argument for: the field has never fully shed the habit of blaming families for individual distress.

1.3 What "relational" actually means: five claims

"Systemic" is a word students learn to say before they learn to mean. Here are the five claims that make the relational frame a frame, in ascending order of how much they cost you as a clinician.

Claim 1 Behaviour is patterned. Any two people who spend time together generate repeating sequences. The pattern is not in either person; it is in the loop. A couple who report "we never talk" will, within twenty minutes of a first session, demonstrate precisely how the not-talking is produced: one asks a question that contains an accusation, the other answers with a fact, the first escalates the volume, the second goes quiet. Neither is doing it to the other. Both are in it.

Claim 2 **The pattern is maintained by its consequences, not by its causes.** This is cybernetics in one sentence. The question "why did this start?" is usually unanswerable and clinically useless; the question "what keeps it going?" is answerable and clinically decisive. A father's withdrawal is maintained by his wife's criticism, which is maintained by his withdrawal. Ten years of history explain how they got into the loop; only the loop explains why they are still in it.

Claim 3 **Meaning is negotiated between people.** The same act means different things to different family members, and those meanings are not private. A daughter's silence means "respect" to her father, "punishment" to her mother, and "safety" to herself. The clinical task is not to find the true meaning but to make the several meanings audible to everyone in the room.

Claim 4 **Symptoms can serve the system.** Not always, not deliberately, and never in a way that makes the symptomatic person responsible. But frequently enough that the therapist must ask: what would change in this household if this symptom disappeared tomorrow? Sometimes the honest answer is "nothing." Sometimes it is "my marriage would have to be looked at," and that answer reorganises the whole treatment.

Claim 5 **The observer is inside the observed system.** You are not a camera. Your presence changes what the family does, what they say, what they notice about themselves, and what they will risk in front of each other. The family that arrives furious will perform its fury for you; the family that arrives polite will perform its politeness. This is not contamination to be minimised it is the instrument you have. Second- order cybernetics, which students meet formally in MFT 715, is simply the discipline of using that instrument knowingly (Figure 1.1).

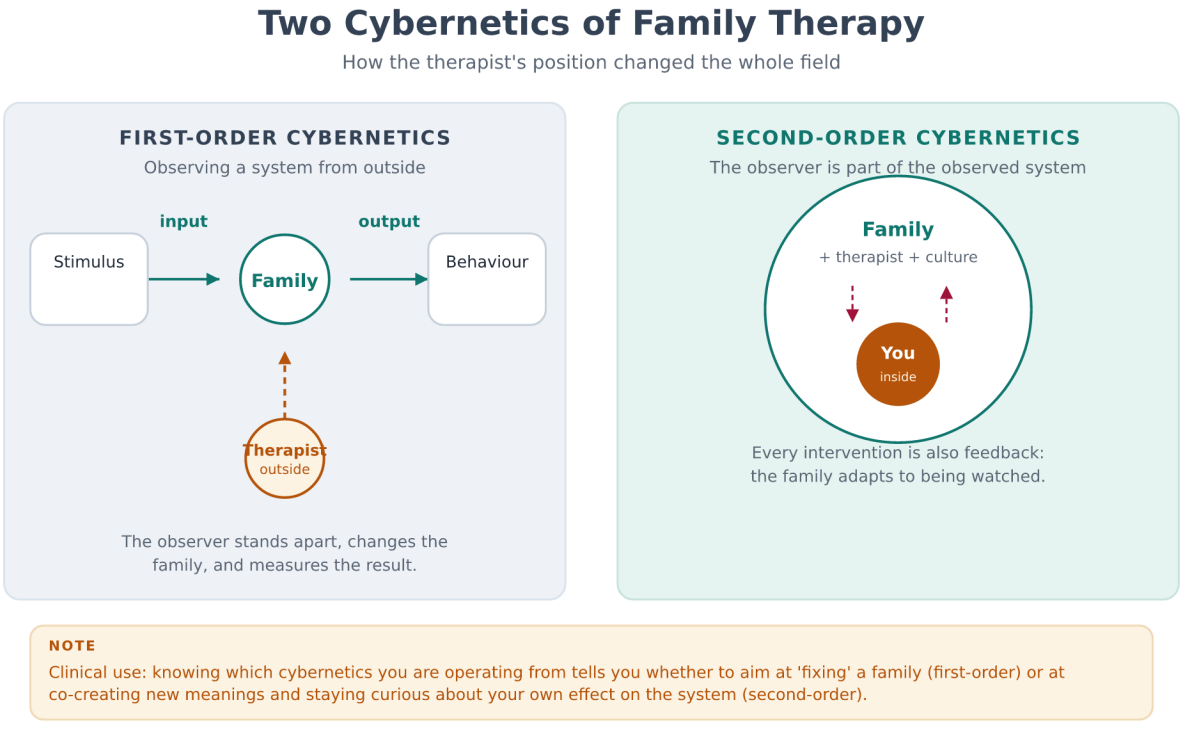


Figure 1.1 First- and second-order cybernetics: where the therapist stands

A working definition. Marriage and family therapy is the assessment and treatment of relational systems couples, families, and the wider networks in which they live using interventions that target interaction patterns, meaning systems, and the emotional bonds between people. Read the definition carefully: assessment of relational systems means the unit of assessment is the relationship; interventions that target patterns means the unit of intervention is the loop, the rule, the structure or the story; emotional bonds means that a purely behavioural account is insufficient, because people are attached to each other and the attachment is what makes the pattern stubborn.

What it is not. Four clarifications, because students routinely collect the wrong ones.

1. **It is not "seeing everyone together" as a matter of policy.** Sometimes the right systemic move is an individual session inside family work. Conjoint work is a means, not a doctrine.
2. **It is not a denial that individuals exist, or that biology, trauma and temperament matter.** It is a disciplined focus on context, not a metaphysics.
3. **It is not the claim that families cause mental illness.** That claim was made in the 1950s and it was wrong, cruel, and expensive. Contemporary systemic practice holds families as the site where distress is expressed, maintained and relieved not as the sole cause.
4. **It is not "we don't do diagnosis."** Systemic therapists diagnose, formulate and often work inside services that require a diagnosis; see MFT 707.

1.4 The four consequences, worked through

The four consequences in the original week note are worth more than a bullet list. Take them one at a time, each with what it changes on Monday morning.

Consequence 1 The unit of assessment changes. You must hear from more than one person, and notice what happens between them. In practice this means the assessment cannot be completed in a single interview with one informant. It means disagreement between informants is data rather than noise: if the mother says the boy is defiant and the father says he is fine, the difference is the finding. It means you watch the room who sits where, who answers for whom, who looks at whom before speaking, what happens to the volume when one person enters.

Consequence 2 The unit of intervention changes. You intervene in sequences, rules, meanings and structures, not only in individual cognition. A cognitive intervention asks the depressed woman to examine the evidence for her belief that she is a burden. A systemic intervention asks what happens in the household in the forty-eight hours after she expresses that belief and often finds that the expression triggers reassurance, that reassurance prevents any change in the division of labour, and that the division of labour is what is depressing her. Both are legitimate. The second is invisible from inside the first.

Consequence 3 The definition of success changes. Success may be a family that argues well, not a family that never argues. This is the hardest consequence for students trained in symptom reduction, because it means the goal is not the absence of conflict but the presence of **repair**. A couple who can fight, rupture and come back together within an

hour is doing better than a couple who never fight and are quietly contemptuous. Outcome measures should reflect this: not only "has the symptom gone?" but "can this system handle disagreement without breaking?"

Consequence 4 The therapist's position changes. You are not a technician outside the system; you are a participant who affects it. Three practical implications. First, you must account for your own effect: the family will behave differently because you are watching, and the difference is informative rather than annoying. Second, you cannot claim neutrality; you can only be transparent about your partiality. Third, your own family of origin is in the room with you which is why every week of this book ends with a reflective task, and why personal therapy and supervision are professional requirements, not luxuries.

> **CASE**
> Mrs Okonkwo brings her 9-year-old son, Ebuka, for "stubbornness". In the first ten minutes she mentions, twice, that his father "is never home". The linear frame says: assess and treat Ebuka's behaviour. The systemic frame says: ask who else is affected by the behaviour, what happens after it, and what would change in the household if it stopped. The second question is not a rejection of the first; it is a widening of the field in which the first can be treated.

1.5 Systems as a way of thinking

Students often assume "system" is jargon for "family". It is not. A system is any set of elements whose interaction produces effects none of the elements produces alone. Ludwig von Bertalanffy's general system theory (1968), Norbert Wiener's cybernetics (1948) and the communications research at Palo Alto gave the field three tools, and they are still the three tools.

Feedback. A system regulates itself through loops. **Negative feedback** reduces deviation and restores the previous state the family that pulls a member back whenever they move too far, the couple whose argument returns to the same sentence. **Positive feedback** amplifies deviation and produces change or escalation the quarrel that grows, the affair that is discovered, the child who leaves. Note that the words are not evaluative: negative feedback is not bad, positive is not good. Most clinical problems are stuck negative-feedback loops with high cost; most crises are positive-feedback loops that have run away.

Homeostasis and morphostasis. Systems resist change in order to remain themselves. When a family comes to therapy, two processes are always running: the pressure to solve the problem and the pressure to keep the system intact. This is why families sometimes seem to sabotage their own treatment, and why the therapist who moves too fast loses the family. The technical answer is to pace: introduce change at a rate the system can absorb, and check that each change is stable before the next.

Rules and punctuation. Watzlawick, Beavin and Jackson's Pragmatics of Human Communication (1967) gave the field its most quoted propositions: one cannot not

communicate; every communication has a content and a relationship level; relationships are defined by punctuation of sequences; humans communicate digitally (words) and analogically (behaviour, tone, gesture). The idea of **punctuation** is the most clinically valuable. Each partner punctuates the same sequence differently: she says "he goes silent, so I get loud"; he says "she gets loud, so I go silent". Both are describing the same loop from different points on it, and each punctuation makes the other the cause. Therapy begins when the loop, rather than either partner, becomes the object of attention.

Structure. Minuchin added what communication theory lacked: an account of organisation. Families have subsystems (parental, sibling, couple), boundaries (clear, diffuse, rigid), hierarchies, alliances and coalitions. A child who is in a coalition with one parent against the other is not "manipulative"; the child is holding a structural position that the adults' conflict has created. Structure gives the therapist something to see in the seating plan and something to change in the session.

Meaning. The later schools Milan, narrative, collaborative pushed the field from pattern to interpretation. If the system is a set of interactions, it is also a set of stories about those interactions. A family that has decided "our son is the problem" has organised its attention, its conversations and its hopes around that sentence. Change the sentence and the interactions move; move the interactions and the sentence loses its evidence.

> **PRACTICAL**
> Choose an ordinary recurring household event: the school run, Sunday lunch, the nightly argument about homework. Draw it as a loop with four to six nodes, each node a behaviour, with arrows showing what follows what. Then mark: where is the negative feedback (what restores the previous state)? Where is the positive feedback (what escalates)? Where does each participant punctuate the loop? Bring the drawing to the lab; you will redraw it after hearing someone else's account of the same event.

1.11a What the evidence says, in one page

Students are often told either that family therapy "works" or that the evidence is contested. Both are unhelpful. Here is what a careful reading of the outcome literature supports, stated with the hedge each claim requires.

Family and couple therapy outperform no treatment, and generally match individual treatment for relational problems. Meta-analyses of couple and family therapy trials report effect sizes in the moderate range roughly half a standard deviation to eight-tenths against no-treatment and wait-list controls, with the larger effects for couple distress, child conduct problems, and adolescent substance use. Effects are smaller and less consistent against credible active comparators, which is what you would expect if the specific model matters less than the common structure of good therapy.

The model explains less of the variance than the alliance. The most robust finding in psychotherapy research, replicated across modalities, is that the therapeutic alliance predicts outcome better than the technical ingredients of any particular school. In couple

and family work the alliance is more complicated than in individual work, because there is more than one alliance to build and they can pull against each other: siding with one partner to engage them alienates the other, and a determined even-handedness can leave both feeling unheard. This is why Week 6 of this book spends two sessions on joining, and why your assessed skill in this course includes your ability to hold two alliances at once.

Common factors do a lot of the work. Client factors (severity, chronicity, social support, motivation), the alliance, the therapist's skill and the expectancy generated by a credible rationale account for a large share of change. The practical implication is not that technique is irrelevant but that technique is delivered through a relationship, and a technique delivered through a poor relationship is inert or harmful.

Dose and dropout matter more than most trainees think. A substantial proportion of families attend one session and never return; the first session is therefore an outcome event, not a preliminary. Retention is predicted by early alliance, by the fit between the therapist's explanation and the family's own understanding, and by practicalities — travel, cost, timing, childcare — that a therapist who ignores them will mistake for resistance.

Evidence is unevenly distributed across problems. Couple therapy for relationship distress, behavioural parent training for conduct problems, family-based treatment for adolescent eating disorders, multidimensional family therapy for adolescent substance use, and systemic work with chronic illness in children have reasonably strong support. Evidence for some popular ideas — mechanical use of paradox, unexamined "dysfunctional family" typologies, polyvagal explanations treated as established mechanism — is much weaker. Learning to distinguish these is part of professional formation, and MFT 705 and MFT 721 teach you to read the literature rather than the marketing.

> **CAUTION**
> Do not let the common-factors literature become an excuse for not learning a model. "The alliance is what matters" is true and is also the sentence most often used by trainees who have not yet learned to do anything. You need a model precise enough to be taught, supervised and evaluated, and a relationship good enough for the model to reach the family. Neither substitutes for the other.

1.6 First contact: three openings, annotated

The relational frame is not announced, it is enacted. Three openings show what it looks like when a therapist begins relationally in the first five minutes — before any genogram, any diagnosis, any technique.

Opening A — from the referred person to the referral system.

> **THERAPIST:**
> Before we start, tell me how this appointment came about. Who first said something needed to change, and who agreed? **Mrs Okonkwo:** The school called me. His teacher said he refuses to write anything. **Therapist:** And when you told your husband, what did he say? **Mrs Okonkwo:** (pause) He said I should handle it. He is not around. **Therapist:** So the school spoke to you, you spoke to your husband, and here you are with Ebuka. Who else in the family knows you are here today?

Note what the therapist has done. She has not asked about the symptom at all. She has mapped the **referral pathway**, which is itself a relational fact: who is worried, who is not, who carries the problem, who was told and who was not. She has also discovered, in four questions, the sentence that will organise the assessment: he said I should handle it. That sentence is not a complaint about a husband. It is a description of a household in which one adult holds all the worry, and a child's symptom sits in the space where the other adult would be.

Opening B from the symptom to the sequence.

> **THERAPIST:**
> Tell me about the last time it happened. Not the worst time the most recent. **Mrs Okonkwo:** Yesterday morning. He would not get out of bed. I called him three times, then I started shouting, then he got up and stood in the bathroom for twenty minutes. **Therapist:** And then? **Mrs Okonkwo:** Then I drove him late and I was angry all day. **Therapist:** What did Ebuka do when you were angry all day? **Mrs Okonkwo:** He was very quiet. He even made me tea.

The sequence is now visible: refusal, escalation, compliance, and then **repair initiated by the child**. That is the loop. The therapist did not need to interpret it. She asked for a specific episode and then asked "and then?" until the loop closed. This is the single most useful micro-skill in the first session: **episode, not opinion.** Opinions produce character descriptions ("he is stubborn"); episodes produce sequences.

Opening C from the individual to the room.

> **THERAPIST:**
> (to Ebuka) Your mum says mornings are hard. If I asked her to describe a good morning, what would she say happens? **Ebuka:** (shrugs) She would be happy. **Therapist:** How would she know? What would she see? **Ebuka:** (long pause) My dad would be there.

Three questions, and the child has produced the formulation. The therapist did not ask "how do you feel about your father's absence?" a question no nine-year-old answers truthfully. She asked a **behavioural question about a third person's perception**, which is easier to answer and much harder to fake. Involving the child as an informant, not a patient, is a structural act: it moves the child out of the identified- patient position and into the family's information system.

What all three openings have in common. Each one moves the unit of attention one step outward — from the person to the pathway, from the event to the sequence, from the individual to the room — and each does so with a question that is answerable. That is the whole craft of a first session in miniature. The student who can do these three moves in the first ten minutes will get a formulation. The student who cannot will get a history.

1.7 The Okonkwo case, fully worked

Referral. Ebuka, 9, referred by his school for "defiance and refusal to write". Six weeks of refusing to complete written work; two episodes of leaving the classroom. Class teacher describes him as "stubborn and rude". Mother reports he is "fine at home, except mornings". Father did not attend.

What the linear frame produces. A differential diagnosis: oppositional defiant disorder versus a specific learning difficulty (writing), versus anxiety, versus an attention problem. An assessment plan: psychometric testing, teacher rating scales, classroom observation, possibly a paediatric review. A treatment plan: behavioural contract, reward chart, parent training in commands and consequences. This is not wrong. It is incomplete in a way that will matter, because the most important information in this family is not about Ebuka's writing.

What the systemic frame adds. Four moves, in order.

Move 1 — the referral pathway. Who is worried? The school, then the mother. Who is not in the room? The father. Who decided that the mother should manage it? The father, according to the mother. The first structural fact: **the parental subsystem is not operating as a subsystem.** One adult holds the worry, the other holds the distance.

Move 2 — the sequence. Mornings: refusal, three calls, shouting, compliance, lateness, a day of maternal anger, evening reparation initiated by the child. The second structural fact: **the child regulates the mother's emotional state.** He absorbs her anger and then repairs it. A nine-year-old is doing the work of a partner.

Move 3 — the function of the symptom. What would change at home if Ebuka got up cheerfully and wrote fluently? The mornings would be calm; the mother's day would be easier; and the father's absence would become a topic rather than a background condition. There is no evidence that anyone wants the child to be symptomatic. There is good evidence that the symptom sits at the exact point where a marital conversation would otherwise occur.

Move 4 — the wider system. School, church, extended family. In this case the maternal grandmother is mentioned twice and the paternal aunt once. In many Nigerian households the grandmother is a co-parent and a court of appeal; leaving her out of the assessment is not neutrality, it is an omission.

The formulation, in one paragraph. Ebuka's refusal to write is maintained by a morning sequence in which escalation reliably produces compliance and in which the child then repairs his mother's distress. That sequence is itself maintained by a household in which one parent carries all the worry and the other is absent, and by a school system that has defined the problem as a defect in the child. The symptom is not caused by the

marriage, and nobody is to blame; but the symptom is located where the marriage would otherwise be discussed, and it will be difficult to resolve without that conversation becoming possible.

The plan that follows. Conjoint sessions including the father (with an explicit invitation framed as expertise rather than blame: "I need your view of what happens in the mornings; your wife and I are working from one account"). A behavioural component for the mornings, because it works and because success builds the parents' confidence in themselves as a team. A separate conversation with the parents about the marriage, held only when both are willing. Liaison with the school to reframe "defiance" as "a problem we are assessing", which reduces the pressure that feeds the loop.

> **TASK**
> Rewrite this case as you would present it in a multidisciplinary team meeting in forty seconds, without using the word "systemic" and without blaming either parent. Then rewrite it for the parents themselves. The two versions should differ in vocabulary, not in content.

1.8 Where the profession sits

Students enter training with a blurred map of the mental health professions. In Nigeria, as in most places, the practical differences matter for referral, for legal standing, and for what you are allowed to do.

Profession	Core unit	Typical training	What they usually do with a struggling family
Psychiatry	The patient, medically	Medical degree, then residency	Diagnose, prescribe, admit; refer for therapy
Clinical psychology	The individual	Doctorate or master's plus internship	Assess, formulate, deliver individual therapy
Counselling psychology	The individual in context	Master's or doctorate	Individual and group counselling, some couple work
Social work	The person in their environment	Degree plus registration	Case management, child protection, family support
Marriage and family therapy	The relationship	Master's or doctorate in MFT	Conjoint, family and couple therapy; relational formulation

The distinctive contribution is not a technique. Clinical psychologists do behavioural couple work; psychiatrists do family meetings; social workers do family support. The distinctive contribution is the **relational formulation** the disciplined habit of locating the problem in the space between people, and the training in interventions that change that space. A therapist who can only see individuals will treat a family by treating its members one at a time and will be genuinely puzzled when the symptom returns.

Settings in which the work happens. Outpatient clinics and private practice; hospitals, where family meetings follow diagnoses that reorganise households; schools, where the referral pathway usually begins; NGOs and humanitarian settings, where family

separation, displacement and bereavement are the presenting context; faith settings, where pastors, imams and church counsellors are often the first and sometimes the only port of call; and courts and child protection services, where the therapist's opinion may be read by a judge. Each setting changes the ethics (see MFT 704), the assessment (MFT 707) and even the language you use.

Scope of practice. Three boundaries worth stating on the first day of training. First, family therapists do not prescribe; any question about medication is a referral question, and the referral should be made without implying that medication is a failure. Second, family therapists do not give legal advice; they provide clinical opinion and, where required, reports, and they tell families to obtain a lawyer (see MFT 724). Third, family therapists do not practise beyond their competence and competence is specific: adult couple work is not child work, child work is not forensic work, and neither is trauma processing without specific training (MFT 711, MFT 714).

> **AUDIT**
> Write down, in one sentence each, what you would do and would not do if a family asked you to (a) advise whether the husband can legally take a second wife, (b) tell the court which parent should have custody, (c) prescribe something for the child's sleep. Bring the three sentences to the lab.

1.9 Culture, faith and class in the first meeting

The relational frame is not culturally neutral, and pretending otherwise is its own kind of arrogance. Three adjustments matter from the first session in most Nigerian consulting rooms.

Kinship is wider than the household. A genogram that stops at the nuclear family will miss the grandmother who raised the child for three years, the uncle who pays the school fees and therefore has standing, the aunt who lives with the family and disciplines the children, and the cousin whose presence has changed the sleeping arrangements. Ask who is involved, not who lives there. The distinction between household and family is one of the most practically useful in the whole programme.

Faith is a resource and a referral system, not a rival. Families commonly arrive having already seen a pastor, an imam, a traditional healer or all three. The therapist's task is not to adjudicate but to coordinate: what did the prayer, the deliverance, the fasting, the herbal treatment do? What was helpful? What was frightening? What does the family believe is happening spiritually? A family that believes the problem is spiritual and a therapist who believes it is relational can still work together if the therapist takes the belief seriously as a meaning in the system, which is exactly what it is.

Class shapes what is possible. A recommendation to reduce working hours is meaningless to a family with two jobs and no childcare. A recommendation to attend weekly sessions is meaningless to a family who travel four hours. A recommendation to "talk to your husband" is meaningless in a household where his mother is the decision-maker. Practical recommendations fail not because families are resistant but

because the therapist prescribed from a life the family does not have. The discipline is to ask, before prescribing: what would have to be true in your week for this to happen? The answer is usually the real treatment plan.

CAUTION

Do not treat cultural deference as consent. A wife who says "I will do whatever my husband decides" may be expressing a genuine value, or may be signalling that she cannot speak safely. The difference is discovered by seeing her alone at least once, and by asking about safety in the standard way not by assuming either interpretation.

1.10 Six errors that begin in week one

These are the mistakes students actually make in their first role-plays, and each one is a failure of the relational frame rather than a failure of technique.

1. **The single-informant assessment.** You interview the mother, form a picture, and never test it. Fix: no relational formulation without at least two informants, and no family assessment without asking who is missing.
2. **The character formulation.** "He is oppositional." "She is enmeshed." These are descriptions of persons disguised as systemic language. Fix: every formulation must contain at least one sequence ("when X, then Y, then Z") and at least one context ("in the presence of ").
3. **Premature reassurance.** "I'm sure he loves you." It closes the exploration and hands the therapist's opinion to a family that needs its own. Fix: acknowledge the experience, ask for the evidence, keep the question open.
4. **The alliance slide.** You begin to like one member more than another and your questions begin to build the case for that person. Fix: track your questions across a session (record it) and count how many were addressed to each person and whose account each question elaborated.
5. **The technique-first session.** You arrive with a genogram to draw, a technique to demonstrate, a model to apply and the family's actual concern is never heard. Fix: the first session belongs to the family's account, not to the therapist's plan.
6. **The safety omission.** You conduct a full conjoint assessment without ever asking about violence. Fix: every first session includes a private screen for harm to self, harm to others, abuse of children and intimate partner violence, conducted in a way that makes disclosure possible (see MFT 704 and MFT 712).

REFLECT

Which of the six are you most likely to commit? Not in theory: in your last conversation with someone who was struggling. Write the answer down. You will be asked to revisit it in Week 14.

1.11 Laboratory: the widening question

The week's practical is a triad drill in which the therapist may only ask questions that include another person. It looks simple and it is not: most students discover that their natural question is a question about an individual, and that widening feels artificial for the first twenty minutes.

Setup. Groups of three. Roles rotate every eight minutes, so each person is therapist once. One person brings a real but non-clinical difficulty a disagreement with a flatmate, a difficult decision about a sibling, a strain with a colleague. No clinical material, no disclosure that would be regretted.

Rules for the therapist.

1. No statements, interpretations or advice questions only.
2. Every question must include a third party or a relationship: "who notices ", "what does your sister do when ", "who would disagree ", "what happens between you after ".
3. One question at a time, with silence after each.
4. Eight minutes, then stop whether or not the conversation feels finished.

Observer's task. Two columns on one sheet: **W** (questions that widened the field) and **N** (questions that narrowed it back to the individual). Tally, then quote the single most useful widening question and the single most narrowing one.

Debrief questions. What did the client notice about their own description when it was widened? Which widening felt intrusive, and why? What did the therapist want to say but could not? What happened to the client's tone when a third person entered the story?

Marking rubric (used in the Week 6 skills gate).

Criterion	Weight	What full marks looks like
Proportion of widening questions	30%	More than 80% of questions include another person or a relationship
Question quality	25%	Open, specific, answerable; no double questions; no embedded advice
Pacing and silence	20%	Roughly one question per 30 45 seconds; client speaks more than therapist
Use of the client's words	15%	At least three questions reuse the client's own phrasing
Reflexivity in debrief	10%	Can name what was hard, what was tempting, what they avoided

> **DRILL**
> Record a five-minute role-play on your phone. Count your questions. Then count how many contain a third person. Most first attempts are around one in ten. The target is eight in ten, and it takes about three hours of deliberate practice to get there.

1.12 Reflective task

Write 500 words on the last significant conflict in your own family, then rewrite it from the point of view of the person you understood least. Note what changed in your body as you wrote. Keep both versions; you will return to them in Week 14, when you write your emerging clinical stance, and you will find that the second version contains the therapist you are becoming.

Two rules for this task. Do not show it to the person concerned, and do not bring the person's name into the seminar. The purpose is to develop the capacity to hold more than one punctuation of the same sequence at once which is precisely what you will ask families to do.

1.13 Self-test

Six questions, answers below. If you cannot answer 4 and 5 without notes, reread 1.5.

1. Define negative and positive feedback, and give one clinical example of each.
2. Why is "punctuation" clinically useful?
3. State the double-bind hypothesis and its current scientific status.
4. What is symptom substitution, and what does it imply for treatment?
5. Name the five claims of the relational frame, in order.
6. What are the four consequences of treating the relationship as the client?

Answers. 1. Negative feedback reduces deviation and restores a prior state (a family pulling a member back from independence); positive feedback amplifies deviation (an argument that escalates through counter-complaint). 2. Because each participant's punctuation makes the other appear to be the cause; the therapist who makes punctuation visible turns blame into a loop both can act on. 3. A person receives contradictory messages at different logical levels, cannot comment on the contradiction and cannot leave the field; it is historically important and not a supported causal explanation of schizophrenia. 4. The observation that removing a symptom is often followed by a new symptom elsewhere in the system; it implies that treatment must change the system, not only the symptom. 5. Behaviour is patterned; patterns are maintained by consequences rather than causes; meaning is negotiated; symptoms can serve the system; the observer is inside the observed system. 6. The unit of assessment, the unit of intervention, the definition of success, and the therapist's position.

1.14 Readings for this week

Read the first two before the lecture; the rest are for the assignment.

- **Bateson, Jackson, Haley & Weakland (1956)**, "Toward a theory of schizophrenia", Behavioral Science. Read it as a historical document and notice how carefully the authors hedge.
- **Watzlawick, Beavin & Jackson (1967)**, Pragmatics of Human Communication, chapters 1 2. The axioms, in the original.

- **Nichols & Schwartz**, Family Therapy: Concepts and Methods, chapter 1. The best single overview of the field's history.
- **Becvar & Becvar**, Family Therapy: A Systemic Integration, chapters 3 5. On cybernetics and second-order thinking.
- **Hoffman (1985)**, "Beyond power and control", Family Systems Medicine. The turn to second-order cybernetics, written by a participant.
- **Minuchin (1974)**, Families and Family Therapy, chapter 2. Structure, in Minuchin's own voice.

1.6a An extended transcript, annotated

Role-plays teach faster than principles. Below is a twelve-turn excerpt from a first session with a couple who present with "we cannot communicate". The therapist is a trainee in her second term. Read the transcript, cover the annotations, and decide what you would have asked next. Then read the annotations.

> **T1 Therapist:** What would you like me to understand first?
> **T2 Folake:** He doesn't talk to me. He comes home, eats, watches television, sleeps. I am alone in this house.
> **T3 Therapist:** (to Dele) What do you think your wife means when she says she is alone?
> **T4 Dele:** I don't know. I work. I am tired. She is not alone, there are three children in the house.
> **T5 Therapist:** Folake, what do you do when he is watching television?
> **T6 Folake:** I stand there. Sometimes I ask him something and he says "mm".
> **T7 Therapist:** And what happens then?
> **T8 Folake:** I get angry. I say something sharp. Then he goes to bed.
> **T9 Therapist:** Dele, when she says something sharp, what goes through your mind?
> **T10 Dele:** That nothing I do is enough. So I stop.
> **T11 Therapist:** And when you stop, Folake, what happens to the anger?
> **T12 Folake:** It grows. And then I cannot sleep and I am tired the next day and the children see me crying.

Annotations.

T1 is a neutral opening that hands the agenda to the couple without inviting either partner to be the complainant. Compare "What is the problem?" which usually produces one person's problem statement and positions the other as the problem.

T3 is the first systemic move: instead of asking Dele to defend himself, the therapist asks him to **interpret his wife**, which requires him to engage with her experience rather than his own. It also reveals whether he has a theory of her mind at all. He does not: "I don't know" is honest and important.

T5 is an **episode question** and it is doing three things at once. It bypasses the argument about whether she is alone (unwinnable), it specifies a moment (television, evening), and it puts the therapist physically in the room with them. Students who ask "why do you feel

alone?" get an explanation; students who ask "what do you do when he is watching television?" get the behaviour that maintains the loneliness.

T7, "and then?", is the single most productive two-word intervention in family therapy. It builds the sequence one link at a time and stops the therapist from doing the formulating.

T9 takes the same event and asks for the **other partner's internal sequence**, which turns two monologues into one loop. Note that the therapist does not challenge Dele's account ("three children in the house" is not an answer to loneliness, but arguing about it would lose him). She lets the sequence do the work.

T10 is the pivot of the session. "Nothing I do is enough, so I stop" is a complete formulation of Dele's side of the loop, offered by Dele. Nothing the therapist could have said would have been as convincing or as usable.

T11 closes the loop by returning to Folake with the consequence of his withdrawal. At T12 she supplies it: the anger grows, sleep is lost, the children see her crying. The loop is now on the table, described entirely in the couple's own words, and neither partner has been blamed.

What the therapist has **not** done in twelve turns is worth listing, because trainees over-value these moves: she has not asked about the history of the marriage, has not asked how they met, has not asked about sex, has not offered an interpretation, has not normalised, has not asked either partner to change anything, and has not used the word "communication". Every one of those is legitimate; none of them was needed to establish the relational frame in the first ten minutes.

> **PRACTICAL**
> In pairs, take turns being the therapist with this transcript. Cover T3 to T12 and improvise your own next question at each point. Then compare with the original. Where did you ask for an opinion rather than an episode? Where did you reassure? Where did you take a side without noticing?

1.10a Three punctuations of one quarrel

The same sequence can be punctuated at least three ways, and each punctuation licenses a different intervention. Take the quarrel the couple above actually have.

Punctuation 1 the character story. "He is cold and withdrawn; I am affectionate and he rejects me." What it licenses: an appeal to Dele to change, a diagnosis of Dele (avoidant, depressive, possibly autistic), and a treatment that works on Dele. What it costs: Folake's contribution disappears, Dele becomes a project rather than a partner, and Dele who already believes nothing he does is enough experiences therapy as one more arena in which he fails.

Punctuation 2 the mirror story. "She criticises; I withdraw; she criticises more." What it licenses: a circular formulation, a sequence map, and interventions aimed at the loop (interrupting the escalation, slowing the exchange, rehearsing a different response to

criticism). What it costs: the loop can become an abstraction that hides the asymmetry of power and the real losses his hours, her sleeplessness, the children's exposure.

Punctuation 3 the meaning story. "She is asking for evidence that she matters; he is protecting himself from evidence that he fails." What it licenses: work on attachment, on the meanings each assigns to the other's behaviour, and on the moment of turning toward or away. What it costs: meaning work without behaviour change can become insight that never reaches the household.

A competent therapist holds all three and chooses deliberately, usually sequencing them: the loop first (because it lowers blame), the meaning second (because it explains why the loop is so adhesive), and the character story never as a formulation, only as each partner's experience to be listened to. This pluralism several punctuations available, one chosen for a reason is what distinguishes a systemic clinician from someone who has memorised a model.

> **DEBATE**
> "Circular formulations are kinder but less true." Fifteen minutes. Note that the strongest argument for circularity is not moral but empirical: linear accounts of complex interaction are almost always wrong about causation, even when they are right about blame.

1.13a Examination question with model answer

Question (20 marks). "Family therapy is a way of seeing, not a set of techniques." Discuss, with reference to the historical development of the field and to at least one clinical example.

Model answer, in note form, with the markers' expectations visible.

Introduction (2 marks). State the claim in your own words: the relational frame is an epistemological commitment a claim about the unit of analysis and the nature of causation and techniques are downstream of it. Signal the structure of the answer: history, epistemology, clinical example, limits.

History (6 marks). Three research programmes converged on the same failure: Bateson's group and the double bind; Lidz and Wynne on family organisation around psychosis; the child guidance movement and symptom substitution. The aetiological claims of the 1950s were largely wrong and you should say so; what survived is the methodological insight that sequences carry information individuals do not. Trace the schools as answers to "what is the unit?" Palo Alto (the sequence), Bowen (the generations), Minuchin (the structure), Milan (the meaning), Milwaukee (the solution), Adelaide (the story).

Epistemology (6 marks). Circular versus linear causality; feedback and homeostasis; punctuation; second-order cybernetics and the therapist's position. Use Watzlawick's axioms precisely content and relationship levels, digital and analogic communication and show what each buys clinically. Demonstrate that you understand the difference between an ecological claim (the system is the unit of assessment) and a causal claim (the system causes the symptom), and reject the second.

Clinical example (4 marks). The strongest answers use one case throughout rather than several in passing. The Okonkwo case works: a nine-year-old's school refusal, a morning sequence, a parental subsystem that is not operating, and a formulation that no single-informant assessment could have produced. Show what the relational frame let you see and what it let you do.

Limits (2 marks). Name them honestly: the frame can obscure individual biology, trauma and temperament; it can be used to distribute responsibility in ways that leave an abusive member unaccountable; it can become a language that describes loops without ever changing them. Note the feminist critique of neutrality and the evidence that the alliance not the model predicts much of the outcome.

Common ways to lose marks. Describing techniques without explaining the frame; treating "systemic" as a synonym for "family"; repeating the double-bind hypothesis as if it were current; failing to give a clinical example at all; writing about circular causality without ever defining feedback.

1.13b What "good" looks like at the end of week one

A checklist you can use on yourself before Week 2. Tick only what you can do without notes.

- Explain, in ninety seconds, why the field moved from the individual to the relationship, using three historical examples and one honest caveat about the science of the 1950s.
- Define negative and positive feedback and give a clinical example of each that you have actually seen.
- Explain punctuation to a couple without using the word "punctuation".
- Turn three opinion questions into episode questions, out loud, without hesitation.
- State the four consequences of treating the relationship as the client, and give one practical implication of each for a first session.
- Describe the difference between a linear and a circular formulation using the same case twice.
- Name the three professional boundaries from 1.8 and what you would say to a family that crossed them.
- Ask about safety in a first session without it sounding like an accusation or an afterthought.

Week 2 Linear and circular causality

2.1 The lecture in ninety minutes

Last week asked who is the client. This week asks a harder question: **what kind of thing is a cause?** It is the single most consequential idea in the programme, because everything downstream the formulation, the questions you ask, what you count as progress, how you write a report for a court depends on whether you think in lines or in loops.

Students usually arrive believing they already understand this. They can define circular causality in a sentence: "each person's behaviour causes the other's." Then they conduct a role-play in which every question they ask is a search for who started it. The gap between being able to say it and being able to do it is the work of this week.

Minutes	Segment	What the lecturer does	What students do
0 10	Two sentences	Reads a case in a linear account, then in a circular account	Mark every causal verb in both
10 25	The philosophy nobody teaches	Aristotle's four causes, Hume, Mills, why "cause" is plural (2.2)	Sort five clinical explanations into cause types
25 45	Feedback, formally	Loop diagrams, delay, gain, thresholds (2.3 2.4)	Draw the same family as two different loops
45 60	Punctuation, again	The same quarrel punctuated three ways (2.5)	Identify their own punctuation habit
60 75	Circular questioning	Live demonstration, then triads (2.6)	Ask three circular questions each
75 90	Hypothesis discipline	How to hold a hypothesis without becoming its servant (2.7)	Write one hypothesis, then one disconfirming question

The lecture has a deliberate shape: philosophy first, engineering second, clinical craft third. Students who skip the philosophy conclude that circular causality is a nicer way of saying "we are all responsible". Students who skip the engineering cannot tell a loop from a list.

2.2 What a cause is: four traditions you will use without knowing

Family therapy inherited a muddle about causation and never fully cleaned it up. The muddle is worth untangling, because most of the field's disagreements about evidence, responsibility and blame are disagreements about what counts as a cause.

Aristotle's four causes. Aristotle distinguished the material cause (what something is made of), the efficient cause (what set it in motion), the formal cause (the pattern it realises) and the final cause (the purpose it serves). A wooden chair: wood, a carpenter, a design, the purpose of sitting. Now apply the scheme to a symptom. A boy's school refusal: his anxiety (material), a frightening event (efficient), a pattern of family accommodation (formal), and the function the refusal serves in the household (final). Most clinical arguments are people defending one of these four while denying the others. The systemic tradition is chiefly interested in the formal and final causes the pattern and the function which is exactly why it seems to slight the efficient cause (the event that started it) and infuriates colleagues who want to know what happened.

Hume and the regularity theory. A cause is an event constantly conjoined with another, temporally prior, and contiguous with it. This is the model implicit in most medical training and in most of what patients believe. It works well for infections and badly for families, because in families the "effect" is usually also the "cause", arriving before and after in alternation.

Mill's methods. Agreement, difference, concomitant variation. These underwrite the randomised trial, and they are why the trial remains the best tool for the question "does this treatment cause improvement?" They are much weaker for the question "what maintains this pattern in this family?", where the relevant variation cannot be manipulated.

Systems and constitutive causation. In a system, the state of each element is partly constituted by its relations to the others. A thermostat is not caused to switch by the temperature in the way a billiard ball is caused to move by another; the switching is what the system does. Family therapists use this sense when they say that the pursuit and the withdrawal "cause" each other: not that A pushes B and B pushes A in a chain, but that the two behaviours are what they are in virtue of the loop they form.

> **EXERCISE**
> Take the sentence "his drinking broke up the marriage". Rewrite it four times, once for each Aristotelian cause. Then decide which of your four sentences a court would want, which a wife would want, and which would actually help in session.

Two consequences follow, and students should write them down.

First, circularity is not a moral claim. "We are both in the loop" does not mean "we are equally responsible", and it certainly does not mean "the victim is complicit". A man who is violent and a woman who placates him are in a loop, and only one of them is doing the violence. Confusing description with responsibility is the most serious error in the systemic literature, and the feminist critique of the 1980s developed by Hare-Mustin, Goldner, Bograd and others was right about it. The corrective is not to abandon circularity but to pair it with an explicit account of power: who has the resources, who can leave, who is believed, whose account is treated as the account.

Second, circularity is a stance, not a finding. You cannot observe a loop from outside; you can only offer a description of a loop from a position inside it. That is second-order cybernetics in one line, and it means your circular formulation is a hypothesis with an author, not a fact with a discoverer.

2.3 Feedback, formally

The engineering vocabulary is not decoration. It gives you words for things you can otherwise only gesture at, and the words make the difference between a formulation that guides intervention and one that merely sounds sophisticated.

Loop, not line. A loop has a variable, a sensor, a comparison with a reference value, and an effector. In a family: the variable might be proximity between the couple; the sensor is each partner's attention to the other; the reference value is an implicit standard of how close they should be; the effector is the behaviour that moves them. This sounds mechanical and is not: the reference values are moral and historical, inherited from the families the partners grew up in.

Negative feedback. Deviation is detected and reduced. The system returns to its set point. Clinically: the family that pulls the adolescent back whenever she separates; the

couple whose argument always returns to the same unresolved sentence; the mother whose anxiety rises when the household is calm, and who finds a problem to restore the familiar tension. Note the crucial point: **negative feedback produces stability, and stability is not always health.**

Positive feedback. Deviation is amplified. Clinically: the argument that escalates because each response is experienced as an attack; the affair discovered, denied, discovered again; the child's school refusal that produces maternal anxiety that produces more refusal. Positive feedback produces change, and change is not always growth. Most crises are runaway positive loops; most chronic presentations are locked negative loops.

Delay. The most under-taught concept in the field. Systems respond to yesterday's state, not today's. A wife raises a complaint on Monday; the husband responds on Thursday with a small act of help; she has by then concluded he does not care and does not notice the help. Delay is why good interventions often look as though they failed: the change is in the pipeline, and the therapist who panics at session four and changes approach destroys the very feedback they needed to see. Rule of thumb: **give a structural intervention three sessions before judging it.**

Gain. How much a system responds to a small deviation. High-gain systems escalate fast and are fragile; low-gain systems are stable and can be rigid. A high-gain couple needs the therapist to slow the exchange and lower the volume; a low-gain family needs the therapist to introduce novelty, raise the emotional temperature slightly, and use enactment. This is the practical payoff of the vocabulary: gain tells you whether to accelerate or decelerate.

Thresholds and hysteresis. Systems do not change smoothly; they flip. A family tolerates a behaviour for years and then, at a threshold, reorganises around it the daughter's disclosure that triggers a family schism, the debt that ends the pretence of solvency. Hysteresis means the threshold for going one way is not the threshold for coming back: it is easier to enter a crisis than to leave it, and families often need help to cross back.

> **PRACTICAL**
> Draw one family twice. First as a negative-feedback loop with a labelled set point: what state does this family return to, and how does it get there? Then as a positive-feedback loop: what is amplifying, and what would a threshold for change look like? Ten minutes, both drawings, no words allowed beyond labels.

2.4 Three ways of drawing the same family

The Adeyemi family: Amara, 16, has refused school for six weeks. Her mother has depression and is on medication. Her father was made redundant fourteen months ago and drinks in the evenings. There is a younger brother, 11.

Drawing one the linear account. Redundancy paternal drinking marital conflict maternal depression Amara's caregiving and anxiety school refusal. Every arrow points one way. This drawing is true as far as it goes, and it is what a referral letter

usually contains. It is also useless as a treatment plan, because every arrow points at a cause the therapist cannot move, and the only intervention it licenses is at Amara.

Drawing two the circular account. Amara's refusal keeps her at home; her presence at home reduces the parents' opportunity to argue; the reduction in argument lowers the mother's distress; the mother's lowered distress makes Amara's presence feel necessary; the father's drinking continues in the space that the parental conflict has vacated. No single arrow is the cause; each arrow maintains the next. This drawing is also true, and it is actionable: the loop has four places where it could be interrupted.

Drawing three the power account. Overlay resources and legitimacy: who earns, who decides, who is believed, who can leave, whose distress is taken seriously. The father's drinking is not neutralised by being "part of a loop": it is a behaviour with a cost, exercised by the person with the most structural power in the household. The mother's depression constrains her capacity to act. Amara has the least power and the most symptomatic expression. The overlay changes what the therapist says: the mother is not "enmeshed", she is exhausted and unsupported; the father is not "in a loop", he is a man who is drinking and needs to be asked to stop, with the consequences named.

What the three drawings teach. A formulation is a choice of drawing for a purpose. The linear drawing is good for history and for medico-legal chronology; the circular drawing is good for intervention design and for lowering blame; the power drawing is good for ethics, safety and responsibility. A competent clinician keeps all three and says which they are using and why. A weak clinician uses the circular drawing because it is the house style and then cannot understand why the wife in the violent marriage is not helped by a conversation about mutual escalation.

> **CAUTION**
> If there is violence, the circular drawing must never be used in the room as the primary formulation until safety has been established and responsibility assigned. Nothing in this week's material licenses the sentence "you are both keeping the cycle going" to a woman who has been hit. See MFT 704, MFT 712 and MFT 714 for the full protocol.

2.4a A worked loop, with numbers

Circular accounts are often criticised for being unfalsifiable. They need not be. Any loop can be given rough magnitudes, and doing so changes what you notice.

Take Loop B from the Adeyemi case: mother's low mood Amara's caregiving mother's temporary relief Amara's increased responsibility Amara's anxiety about school school refusal more time at home.

Assign a plausible scale, in the family's own terms, and mark how confident you are.

Link	Direction	Rough magnitude	Confidence	Evidence
Mother's mood Amara's caregiving	caregiving as mood falls	Large	High	Both describe it; it happens most days

Link	Direction	Rough magnitude	Confidence	Evidence
Caregiving mother's relief	Relief for hours	Moderate	High	Mother: "she settles me"
Relief Amara's load	Load grows, no reduction	Moderate	Medium	Described across six weeks
Load anxiety about leaving	Anxiety rises	Large	Medium	Amara: "I can't settle at school"
Anxiety refusal	Refusal	Large	High	Directly observed
Refusal time at home	Six extra hours a day	Large	High	Arithmetic
Time at home caregiving	Loop closes	Large	High	Session observation

Three things follow, and each is a clinical decision rather than a theoretical one.

First, the loop has a strongest link. The largest, best-evidenced arrows run through time at home. That is why the first intervention addressed the timetable not because school attendance is a moral good, but because it is the highest-leverage node in the loop as drawn.

Second, magnitudes tell you what to expect from partial change. If caregiving fell by half but the mother's mood did not improve, the loop would reassert within days, because the relief arrow would still be live. This is why single interventions on complex loops fail and why therapists conclude, wrongly, that "the family is resistant". The family is not resisting; the arithmetic is against you.

Third, confidence is part of the formulation. Two of the seven links are medium-confidence. A good therapist names them in supervision and tests them with a question, rather than building a plan on them silently. "I am assuming your daughter's being at home makes your days easier; is that right, or is it more complicated than that?" is not a weak question it is the question that protects the plan.

> **PRACTICAL**
> Take your own loop drawing from 2.3 and add two columns: magnitude (small/moderate/large) and confidence (low/medium/high). Then mark the single link you would test first in the next session, and write the question you would use to test it.

2.5 Punctuation, worked

Punctuation is the process by which each participant marks a beginning in a continuous sequence, thereby assigning cause. It is not a mistake to be corrected; it is the ordinary operation of a mind trying to make sense of a loop it is inside.

The four standard punctuations in couple work.

1. **The complainant's punctuation.** "I complain because he withdraws." The complaint is a response; the withdrawal is the cause. Intervention: ask the complainant what they do immediately before the withdrawal, in a specific episode.

2. **The withdrawer's punctuation.** "I withdraw because she criticises." The withdrawal is a response; the criticism is the cause. Intervention: ask what they notice in their body immediately before they go quiet, and what they are protecting.
3. **The reciprocity punctuation.** "We wind each other up." This sounds enlightened and often conceals the asymmetry: someone is doing more of the escalating, or someone is more afraid.
4. **The system punctuation.** "The loop between us has a life of its own." Useful therapeutically, but it can become a way of talking about the relationship at the exact moment when someone needs to be accountable for a specific act.

How punctuation is changed in the room. Not by argument. Telling a husband "actually you both do it" does not change his punctuation; it recruits him into defending it. Punctuation changes through **specific episode work**: slow the sequence down until both partners can see the moment where their own action occurred, and let them describe it themselves. In practice this means three moves, which students should rehearse until they are automatic:

Move 1 narrow the time frame. "Not the usual argument. The last one. Tuesday evening. Start at the point where you were both still in the kitchen."

Move 2 ask for the moment before. "What were you doing in the thirty seconds before he stood up?" This is the question that gives the complainant back their own agency without blaming them, because it is factual rather than evaluative.

Move 3 ask what they noticed inside. "What went through your mind when she said that?" This converts a behavioural sequence into an emotional one, which is where change becomes possible.

Done well, the couple arrives at a sentence neither could have said at the start: "I go quiet because I think I have already lost, and then she gets louder because the quiet looks like contempt." That sentence is the treatment target for the rest of the work.

> **DRILL**
> Record a five-minute role-play in which one partner describes a recurring argument. Listen back and count how many times the therapist asks "why" (which invites justification) versus "what happened next" and "what did you notice" (which invite sequence). Targets: no more than one "why" in five minutes; at least six sequence questions.

2.6 Circular questioning

The Milan associates Mara Selvini Palazzoli, Luigi Boscolo, Gianfranco Cecchin and Giuliana Prata developed **circular questioning** in the late 1970s as the operational form of systemic thinking: questions designed to elicit differences and relationships between people, rather than facts about individuals. The technique is more than fifty years old and remains the most portable skill in the field, because it works inside any model.

The four families of circular question.

1. **Difference questions.** "Who is most upset by this? Who is least? Who would notice first if it changed?" These map the system's distribution of concern and give every member a position.
2. **Behavioural-effect questions.** "When she cries, what does your father do? And then what does your mother do?" These build the sequence in the room, from several points of view.
3. **Comparison questions.** "Is this worse than it was when your mother was alive? Better than last year?" These locate the problem in time and reveal the family's own baseline.
4. **Hypothetical or future questions.** "If this problem disappeared tomorrow, what would be the first thing each of you noticed? Who would miss it?" The second half of that question is the one that produces the information, and it must be asked gently.

Two more devices worth knowing. The **interviewing-the-interviewer** move the therapist asks one member to comment on what the therapist has been asking, which reveals the observing system inside the family. And the **positive connotation** describing the symptom as serving the family's cohesion which was Milan's way of preventing the family from defending itself against the therapist. Both are powerful and both are easy to misuse: positive connotation applied to violence is a serious error, and interviewing-the-interviewer can feel like a parlour trick if it is not clearly in the service of the family's question.

What makes a circular question good. It is answerable; it concerns a third party; it asks for a difference; and it is asked without a preferred answer. The last criterion is the hardest. A question like "don't you think your mother worries more than your father?" is a statement with a question mark, and families can hear the difference even when students cannot.

> **PRACTICAL**
> In triads, one person describes a family difficulty. The "therapist" must ask eight questions, two from each family of circular question. The observer records which questions produced new information and which produced a repeat of information already given. The correlation with quality is brutal and instructive.

2.6a Reframing: the intervention that follows from circularity

: the intervention that follows from circularity

If the loop is the unit, then the fastest way to change a loop is to change the meaning each member assigns to the other's behaviour, because meaning is what selects the response. **Reframing** is the deliberate provision of a new frame that fits the same facts and supports a different response. It is the most portable intervention in the field and the easiest to do badly.

The structure of a reframe. Three parts: the behaviour as observed (unchanging, factual), the intention or function (new, plausible, benign), and the consequence for the

relationship (which now invites a different response). Example, with a father who works late:

> "You are the person who makes sure this family is provided for, and you do it by being away from it. That has worked for the family's security. What your wife is telling me is that it has also taught your son that a man's place is outside the house. Neither of you wants that lesson."

Note what has happened. The behaviour is unchanged: he works late. The intention is newly stated and creditable: he provides. The consequence is newly visible: the son is learning something. The father is not accused and not excused, and a different response is now available to him.

Five rules for a reframe that works.

1. **It must fit the facts.** A reframe the family experiences as false is worse than no reframe: it damages credibility and closes the topic. If you cannot find a genuinely plausible benign intention, do not invent one use a difference question instead.
2. **It must be delivered as an offer, not a verdict.** "I wonder whether " outperforms "What is really happening is " in almost every family, because it leaves room for the family to correct you, which is itself engagement.
3. **It must be addressed to the person whose behaviour is being reframed, in their presence.** Reframing someone to their partner behind their back is gossip with a therapeutic vocabulary.
4. **It must not absolve harm.** You can reframe the function of a behaviour without reframing away its cost. "He drinks to manage a shame he cannot speak" is a reframe; it does not make the drinking acceptable, and the next sentence must say so.
5. **It must be followed by a task.** A reframe without a next step is a compliment. The loop changes when the family does something different, not when it understands something new.

Reframes that students overuse. "You are both doing the best you can" (true and inert). "Your son's behaviour is a message of love" (often false, and it places an impossible burden on the child). "This is a normal stage" (sometimes true, and sometimes a way of not assessing). Each of these is a sentence that makes the therapist feel kind and the family feel handled.

> **DRILL**
> Write three reframes for the Adeyemi case: one for the mother's depression, one for the father's drinking, one for Amara's refusal. For each, check the five rules explicitly, then read it aloud to a peer who plays the family member and tells you whether it landed as an offer or a verdict.

2.7 Hypotheses: how to hold one without being owned by it

A **systemic hypothesis** is a provisional explanation of how the presenting problem is maintained by the relationships in the system, stated so that it can be wrong. Milan's team formalised the discipline: the team forms a hypothesis before the session, tests it through circular questioning, and revises it, keeping several alive at once.

Three properties of a usable hypothesis.

1. **It is relational.** It mentions at least two people and at least one sequence: "when the father criticises, the mother intervenes, and the son then refuses to speak which returns the couple to a familiar division of labour."
2. **It is testable in the room.** There is a question whose answer would make you doubt it. If no such question exists, you are holding a belief, not a hypothesis.
3. **It includes the function.** What work does the symptom do for the system? The answer may be "very little, and the family is simply suffering", which is a legitimate finding and should be said out loud.

The hypothesis-discipline drill. Write your hypothesis in one sentence. Then write three questions: one that would support it, one that would complicate it, and one that would refute it. Ask all three. Trainees routinely discover that they only ever ask the first. That habit is the beginning of confirmatory practice, and it is the most common failure of experienced therapists as well as novices.

Hypotheses in a team. Where a reflecting team or a live supervisor is available, the discipline is strongest: the team holds the hypothesis, the therapist stays curious, and the family is spared the weight of being interpreted. Where there is no team, the therapist must build the discipline internally writing the hypothesis down before the session, and reviewing it after, is not bureaucracy; it is the substitute for a second mind.

> **TASK**
> Take the Adeyemi family. Write three competing hypotheses: one structural, one multigenerational, one strategic. For each, name the single question you would ask first and the observation that would make you abandon it. Bring all three to the lab; you will be asked to defend the one you would bet on, then switch.

2.8 The Adeyemi family, worked: from loop to plan

Referral. Amara, 16, out of school for six weeks. Referred by the school counsellor, who writes: "Amara presents with anxiety and possible depression; family reluctant to engage." Mother attends with Amara. Father "working" and unavailable.

Session one, what actually happens. The mother speaks for twenty minutes. Amara sits with her hood up and says four words. The therapist's temptation, named openly in supervision afterwards, was to get Amara talking the standard move, and the wrong one, because it would have confirmed the family's map in which Amara is the patient and the mother is the escort. Instead the therapist asked three questions, in this order.

1. "Before Amara stopped going to school, what was happening in the house?" Mother: a long account of her husband's redundancy, her own illness, the money.
2. "Who else in the family knows how bad things have been?" Mother: nobody. Her sister "would only worry". Her husband "knows some of it".
3. "Amara, when your mother is having a bad day, what do you do?" Amara, first full sentence: "I stay with her."

The sequence is now visible, and it took three questions. The daughter's presence at home is not only a symptom; it is the household's arrangement for managing the mother's distress in the absence of another adult. The school refusal is doing work.

Formulation. Three loops, nested.

- **Loop A (parental).** Father's redundancy shame and drinking mother's depression father's further withdrawal (he experiences the household as a place where he fails) more drinking. This is the primary loop, and it is invisible while the child is the identified patient.
- **Loop B (mother daughter).** Mother's low mood Amara's caregiving mother's temporary relief Amara's increasing responsibility Amara's anxiety about leaving school refusal. This loop is maintained by love and by the absence of another adult, not by pathology.
- **Loop C (family school).** School pressure mother's shame pressure on Amara Amara's withdrawal school escalation more pressure. This loop is powered by an institution's timetable, which is why it is so hard to slow.

What the therapist did next, and why. The plan was sequenced deliberately, and the sequencing is the teachable part.

Step 1 change the unit of the sessions before changing anything else. The therapist asked for a session with both parents, framed as expertise: "I need your husband's view of what the house is like now; I am working from one account and it is not fair to any of you." Framing the invitation as a request for information rather than a summons to be confronted is the difference between a father who attends and a father who refuses.

Step 2 give the parents a job that is theirs. Not "spend more time together" (an instruction that will be experienced as another failure) but a specific, small, observable task: both parents to handle the next school letter together, before either of them speaks to Amara about it. The task is structural it creates a parental subsystem in the presence of the therapist and it is small enough to succeed.

Step 3 separate the caregiving from the child. With the mother: who else could be with her on a bad day? The answer produced a sister, a neighbour and a church member. The therapist did not ask the mother to need less; she asked the system to supply more adults.

Step 4 take the school out of the loop. A single call from the therapist to the school counsellor, reframing "anxiety and reluctance" as "a family in a difficult year, with a plan", reduced the institutional pressure that was feeding Loop C. Ten minutes of professional-to-professional contact changed the family's week.

Step 5 attend to the father's drinking, on its own terms. Not as a symptom of the marriage. The therapist raised it directly, with the mother present and without moralising: how much, how often, what it does, what he thinks about it. He agreed to a referral for assessment. He did not stop drinking in the course of the work. The family still improved, which is a finding worth holding: improvement is not contingent on every problem being solved.

What happened by session eight. Amara returned to school part-time, with a staged timetable agreed between the parents, the school and the therapist. Her caregiving load was shared. The parents' arguments did not disappear; they became discussable in session, which is a modest and real outcome. The father attended six of eight sessions and was, by his own account, "not fixed, but in the conversation".

What a student should take from the case. Four things. The symptom was doing work, and naming that did not blame the child. The primary loop was the parents', and the presenting problem was the route to it, not a distraction from it. The interventions were sequenced, small, and structural. And not everything was solved which is what real work looks like and what written case studies too rarely admit.

2.9 Annotated transcript: turning a linear account into a loop

The following excerpt is from the first session, minutes 18 26. The mother has been describing Amara's refusal. Read the annotation after each turn.

> **T1 Mother:** She just will not go. I have tried everything. Her father says I am too soft with her.
> **T2 Therapist:** So your husband thinks the answer is firmer handling. What do you think he would say if I asked him why?
> **T3 Mother:** (pause) He would say I let her wrap me round her finger. He thinks I need her at home.
> **T4 Therapist:** What makes you say that?
> **T5 Mother:** Because when she is at home she sits with me. And when she is at school I am on my own all day.
> **T6 Therapist:** So her being at home helps you. And it also worries you.
> **T7 Mother:** (crying) Both. I know it is wrong to say that.
> **T8 Therapist:** It is not wrong. It is the most honest thing said in this room today. Amara, when your mother is crying, what do you do?
> **T9 Amara:** I get her water.
> **T10 Therapist:** And what happens to your own day when you are getting her water?

Annotations.

T2 is the pivot of the whole session and it is worth studying closely. The therapist does not ask "are you too soft?" which would put the mother on trial nor does she defend the mother. She asks the mother to **report her husband's view and its reasons**, which does three things: it brings the absent father into the room as a voice rather than a criticism; it treats the mother as a reliable observer of her marriage; and it reveals that she already

knows what he thinks. Families usually do know. The therapist's job is to make the knowing speakable.

T3 T5 produce the formulation from the mother's own mouth. Note that the therapist asks only "what makes you say that?" five words, no interpretation. The hypothesis (the daughter's presence is serving the mother's isolation) is now the mother's own statement, which is the only form in which it can be worked with. Had the therapist said it, the mother would have had to defend herself against it.

T6 is the **both-and** move: it holds two truths without resolving them. "It helps you and it worries you." This is the linguistic form of circularity, and it is a skill that can be practised: every time you are tempted by "but", try "and".

T7 is the moment of shame, and it must be handled with precision. What the mother fears is that she is being accused of wanting her child ill. T8 does not reassure her ("of course you don't want that") because reassurance would close the opening; it validates the honesty and moves immediately to the child, which distributes the emotional weight and prevents the mother from becoming the identified patient in turn.

T9 T10 bring the child in as an informant about her own behaviour in relation to her mother. T10 is the crucial one: "what happens to your own day when you are getting her water?" It is a question about consequence, addressed to the person with least power, and it invites her to describe her own experience without requiring her to blame her mother. Her answer that she cannot settle, that she worries, that she cannot face school after a night of listening completes the loop.

What the therapist avoided, deliberately. She did not ask about the marriage directly in this session. She did not name the father's drinking. She did not offer a diagnosis. She did not reassure the mother that she was a good mother, though she was. Each omission was a decision: the first session's job was to establish that the problem lived between people, and every one of those moves would have pulled it back into one person.

> **DRILL**
> Take turns reading T1 T5 aloud in pairs. The listener must replace T6 with their own "both-and" sentence. Then replace T8 with a sentence that validates without reassuring. Compare. Most attempts either reassure ("you're a good mother") or interpret ("so you see, you need her at home") both of which close the opening that T7 created.

2.10 Laboratory: the same case, three drawings

The week's assessed practical. Ninety minutes, groups of four, one case per group drawn from the case bank (Appendix to this week).

Stage 1 (20 minutes) linear account. Each group writes the case as a one-directional chain of at least six arrows, in the form "A B C". No loops permitted. This constraint is uncomfortable and instructive: students discover how natural the linear account is, and how much it leaves out.

Stage 2 (25 minutes) circular account. Redraw as a loop. Rules: every node must be a behaviour or an observable event, not a trait; every arrow must be justifiable from something a family member said; at least one node must be something the therapist could actually change.

Stage 3 (20 minutes) power overlay. On the same drawing, mark for each member: resources (income, housing, mobility), legitimacy (whose account is believed), and exit options. Then write one sentence stating what the overlay changes about the plan.

Stage 4 (25 minutes) presenting. Each group presents all three drawings in four minutes. The audience scores on a single criterion: **does the third drawing change the plan?** If the power overlay leaves the intervention identical, the group has decorated rather than analysed.

Marking rubric.

Criterion	Weight	Full marks
Linear account fidelity	15%	Chain is accurate to the referral and contains no smuggled loops
Circular account quality	30%	Nodes behavioural, arrows evidenced, one actionable node identified
Power overlay	25%	Resources, legitimacy and exit named for each member; plan visibly changed
Integration and presentation	20%	Three drawings used together, with the purpose of each stated
Reflexivity	10%	Group can name the drawing they found hardest and why

2.11 Six common errors with circularity

1. **Circularity as absolution.** Using the loop to avoid naming a behaviour that must stop. Fix: pair every circular formulation with an explicit statement of responsibility where responsibility exists.
2. **The endless loop with no entry point.** A drawing with eight nodes and no plausible intervention. Fix: every formulation must nominate the node the therapist will work on first, and why that one.
3. **Loop-washing the history.** "It doesn't matter who started it" is sometimes true and sometimes a way of avoiding a trauma that needs to be named and mourned. Fix: ask the history question, then decide whether to work with it.
4. **Circular questions as interrogation.** Eight questions in a row with no acknowledgement, and a family that closes. Fix: one question, one answer, one acknowledgement; the ratio should feel slow to the therapist.
5. **The hypothesis that cannot lose.** Interpreting every response as confirmation. Fix: write down, before the session, what would count as refutation.
6. **Systems language as a way of sounding clever.** "The system is communicating its homeostatic imperative" is not a sentence that helps anybody. Fix: translate every formulation into language a 14-year-old in the room could follow, and check that you can.

AUDIT

Record one of your own sessions (role-play is fine). Transcribe three minutes. Mark every causal claim you made. Count how many were linear, how many circular, and how many attributed responsibility. If the third number is zero, you have a habit worth breaking.

2.12 Culture, faith and the limits of circularity

Two cautions, both learned the hard way in supervision.

The respectful-withdrawal pattern. In many families, a father's distance is not "coldness" but a culturally scripted division of labour: he is the provider, the mother is the emotional manager of the household, and his non-attendance at a clinic is not a symptom but a role. Reading this pattern as "disengagement" and intervening to increase his emotional involvement can be experienced as an accusation of inadequate masculinity and it will fail. The better move is to reframe the invitation in terms of his existing role: "I need the person who understands the finances and the school fees, because decisions about Amara's school are partly financial." You are not asking him to become someone else; you are asking him to bring what he already does into the room.

The extended-family triangle. Circular accounts built on the nuclear household routinely miss the grandmother who is the actual decision-maker, the aunt who funds the school, or the uncle whose approval the father needs. When these figures are absent from the drawing, the loop looks like a couple problem and the intervention fails at the point where the real authorisation was required. Ask, always: who else would need to agree for this plan to happen? Then decide whether to invite them, telephone them with consent, or work with the family's account of them.

Faith as an explanation in the system. "This is a spiritual attack" is a causal account, and a serious one. It competes with your circular account, and arguing with it is both rude and ineffective. The systemic move is to treat it as part of the system's explanatory map: who in the family believes it, who does not, what the belief licenses (more prayer, less school), and what the family would accept as evidence either way. Very often the family does not need the therapist to agree; it needs the therapist to take the belief seriously enough to plan around it.

CAUTION

Do not use circularity to soften a disclosure. If a father says "she provoked me" about a physical assault, the correct response is not "so you both escalate". It is to name the act, state that it is not acceptable, and follow the safety protocol. Circular thinking is a tool for understanding, never a substitute for safeguarding.

2.13 Key terms for this week

Term	Definition	Why it matters clinically
Linear causality	A causes B in one direction	Good for history and legal chronology; poor for intervention
Circular causality	Each element both causes and is caused by the others	The basis of loop formulations and of blame reduction
Punctuation	The way each participant marks a beginning in a sequence, assigning cause	The mechanism by which blame is distributed; changeable through episode work
Feedback (negative)	Process that reduces deviation and restores a set point	Explains stability, including stable misery
Feedback (positive)	Process that amplifies deviation	Explains escalation, crisis and sudden change
Set point	The state a system returns to	Named in the family's own words whenever possible
Delay	Lag between an action and its effect in the system	The reason therapists abandon good interventions too early
Gain	Responsiveness of a system to small deviations	Tells you whether to slow down or speed up
Threshold / hysteresis	The point at which a system flips, and the asymmetry of flipping back	Explains why crises are easier to enter than to leave
Systemic hypothesis	A provisional relational explanation, stated so it can be refuted	Keeps the therapist curious rather than certain
Circular question	A question that elicits difference or relationship between people	The core skill of systemic interviewing
Difference question	Who is most/least affected; who notices first	Maps the distribution of concern
Behavioural-effect question	What one person does when another does X	Builds the sequence in the room
Positive connotation	Describing a symptom as serving the system's cohesion	Powerful, and dangerous if applied to violence
Second-order cybernetics	The observer is part of the observed system	Makes the therapist's own effect part of the data
Constitutive causation	Relations constitute the elements rather than pushing them	The philosophical basis of the loop

2.14 Evidence note: does circularity survive contact with the evidence?

Students are right to ask whether a conceptual scheme this old still earns its place. Three honest answers.

As a description of interaction, circularity is not controversial. Sequential analysis of couple interaction coding who does what, in what order, and what follows reliably shows reciprocity: negative affect from one partner raises the probability of negative affect from the other, and the strength of that reciprocity predicts relationship satisfaction. Gottman's cascade models and the demand withdraw literature (Christensen

and colleagues) are, in effect, empirical confirmations that the loop is real. What the literature does not support is the stronger claim that the loop is the cause of the distress, or that both parties contribute equally. Asymmetry is the norm: demand withdraw patterns are strongly gendered, with women more often in the demand role and men in the withdraw role, and that asymmetry is not an artefact of observation.

As a therapeutic stance, circularity has modest but real support. Interventions that externalise the problem and reduce mutual blame reframing, positive connotation, circular questioning used to build shared understanding are associated with improved alliance and reduced dropout, and the alliance is one of the strongest predictors of outcome. This is not the same as saying that circular formulations cure. It says that a formulation the whole family can accept is a precondition for the work, and circular formulations are better at earning acceptance than accusatory ones.

As a theory of causation, circularity is under-specified. The critique from outside the field is that "circular causality" often means merely "correlation over time", and the critique from inside is that the metaphor smuggles in a homeostatic assumption that systems prefer the status quo which is not always true and which can make therapists conservative in the face of change that families are ready for. The most defensible position is the one this week has taught: **circularity is a stance for generating hypotheses and questions, not a causal theory to be tested.** Use it to widen the field and lower blame; do not use it to explain the aetiology of anything.

> **DEBATE**
> "Circular causality is a metaphor that has done more good than harm, and it is time to retire it." Twenty minutes. Note that the strongest argument for retiring it is the violence literature; the strongest against is that no better vocabulary for interaction has been found.

2.15 Self-test

Eight multiple-choice items and three short answers. Answers follow.

1. Which statement best captures negative feedback? (a) A process that amplifies deviation (b) A process that reduces deviation and restores a set point (c) A criticism delivered calmly (d) A system's refusal to change
2. "He goes silent because I shout" and "I shout because he goes silent" are examples of: (a) Linear causality (b) Different punctuations of one sequence (c) Homeostasis (d) Positive connotation
3. Which is a difference question? (a) Why do you fight? (b) What do you do when she cries? (c) Who is most upset by this, and who is least? (d) Do you think he is depressed?
4. A couple escalate quickly and the session feels unmanageable. In systems terms the system has: (a) High gain (b) Low gain (c) High delay (d) Strong hysteresis
5. The strongest reason to give an intervention three sessions before judging it is: (a) Families are resistant (b) Delay in feedback loops (c) Therapist fatigue (d) Ethical guidelines

6. Which formulation error does "you are both keeping the cycle going" most clearly risk when there is violence? (a) Over-attributing responsibility (b) Absolving responsibility (c) Confusing content and relationship (d) Ignoring delay
7. A hypothesis that cannot be refuted is best described as: (a) Robust (b) Systemic (c) A belief (d) A difference question
8. Positive connotation should not be used when: (a) The family is angry (b) There is violence or abuse (c) The therapist is unsure (d) The symptom is mild

Answers. 1 b 2 b 3 c 4 a 5 b 6 b 7 c 8 b.

Short answers (150 words each).

Explain the difference between a linear and a circular formulation, using one family. A strong answer names the direction of the arrows, states that the linear account is not false but incomplete, gives a specific sequence, and shows what each formulation licenses clinically including that the linear account is the one a court or a school will usually want.

What does the power overlay add to a circular formulation? A strong answer names resources, legitimacy and exit options; explains that circularity describes interaction while power explains whose interaction counts; and gives an example in which the overlay changes the intervention (for instance, from "reduce mutual escalation" to "name the violence, safety-plan, and refer").

Why can a good hypothesis make a therapist worse? A strong answer describes confirmatory questioning, the loss of curiosity, the family's experience of being interpreted, and the discipline of writing down a refutation condition before the session.

2.16 Reflective task

Think of a recurring difficulty in your own family. Write two paragraphs. In the first, punctuate it the way you usually do that is, with the other person's behaviour as the beginning. In the second, punctuate it from the other person's position, with your behaviour as the beginning. Do not try to be fair; try to be accurate about what each punctuation leaves out.

Then answer, in three sentences: what did you have to believe about yourself to write the second paragraph, and what did you have to give up? Most students find the answer uncomfortable, and that discomfort is the beginning of the capacity to hold a loop without needing to be innocent in it.

2.17 Readings for this week

Read the first two before the lecture.

- **Watzlawick, Beavin & Jackson (1967)**, Pragmatics of Human Communication, chapter 2 punctuation, in the original. Notice how much of the argument is philosophical rather than empirical.

- **Selvini Palazzoli, Boscolo, Cecchin & Prata (1980)**, "Hypothesizing circularity neutrality: three guidelines for the conductor of the session", Family Process. The canonical statement of Milan technique.
- **Bateson (1972)**, Steps to an Ecology of Mind, part III. The cybernetic essays, which are better written than most of what followed.
- **Hoffman (1981)**, Foundations of Family Therapy, chapters 1 4. The best history of the shift from linear to circular thinking.
- **Goldner (1985)**, "Feminism and family therapy", Family Process, and **Hare-Mustin (1978)**, "A feminist approach to family therapy", Journal of Marital and Family Therapy. The critique that circularity without power is not neutral.
- **Christensen & Heavey (1990)** and the later demand withdraw literature. Read one empirical paper on reciprocity and note what it does and does not establish.

Next: Week 3 A short history of the field

Week 3 A short history of the field

3.1 The lecture in ninety minutes

Why does a clinician need history? Three reasons, and they are not decorative. First, models are answers to questions, and you cannot evaluate an answer without the question. Second, the field's current fashions are usually old arguments in new clothes, and a clinician who knows the argument is harder to sell a rebranding. Third, your own professional identity is built from this material: what you call yourself, what you think you are doing, and what you refuse to do all come from a seventy-year conversation.

The lecture is a walk through five waves, with a hard question attached to each.

Minutes	Segment	What the lecturer does	What students do
0 10	Prehistory	The child guidance movement, marriage counselling, social casework	Note which problems each could and could not solve
10 30	Wave 1: communication	Palo Alto, Bateson, Jackson, Haley, Watzlawick	Timeline: place five key texts
30 45	Wave 2: structure and generations	Minuchin, Haley, Bowen, Satir	Draw the same family as each would see it
45 60	Wave 3: meaning	Milan, systemic therapy, positive connotation, circular questioning	Identify what Milan added and what it cost
60 72	Wave 4: solutions and stories	de Shazer, White, Epston, Andersen	Compare two session transcripts
72 85	Wave 5: evidence and integration	Common factors, attachment, neuroscience, feedback-informed work	Argue for what the sixth wave will be
85 90	Set the essay	The comparative assignment (3.14)	Choose two models to compare

3.2 Prehistory: three streams before "family therapy"

Nobody invented family therapy in a single place. Three professional streams were converging by 1950, each with its own methods and its own blind spots.

Child guidance. From the 1920s, clinics in the United States and Britain treated children referred for behavioural and emotional problems. The standard team was a psychiatrist, a psychologist and a social worker: the psychiatrist saw the child, the social worker saw the mother. Fathers were largely absent from the literature, and the mother was usually the object of investigation a framing that did enormous damage and whose echoes are still audible in referrals today ("mother is over-involved"). The practical legacy is the multi-disciplinary team, which survives in child and adolescent mental health services.

Marriage counselling. From the 1930s, and especially after the Second World War, counsellors began seeing couples together often in religious or voluntary settings, often with an explicitly moral vocabulary. The American Association of Marriage Counsellors was founded in 1942. Its legacy is the couple as a legitimate clinical unit and, less happily, a tendency to treat marital distress as a communication problem to be instructed rather than a system to be understood.

Social casework. Caseworkers had always worked with families, but as the environment of an individual client rather than as a system with its own properties. The legacy is attention to context housing, income, kin, migration that purely psychological models lack, and which this library's cultural chapters depend on.

What the three streams lacked. A theory of interaction. Each could describe a person in a family; none could describe the family as a thing with a structure. That arrived from an unexpected direction: engineering and biology.

3.3 Wave 1: communication (1952 1970)

The founding texts are not clinical. They are cybernetic and anthropological, and that origin still shapes how the field thinks.

Bateson's project. Gregory Bateson anthropologist, cyberneticist, and a man whose influence is out of all proportion to the number of patients he treated assembled a research group at Palo Alto from 1952 with Jay Haley, John Weakland and, from 1954, Don Jackson. They studied communication in families with a member diagnosed with schizophrenia, and produced the double-bind paper in 1956. Bateson's wider project was a theory of mind as a property of systems rather than brains: "the difference which makes a difference", information as a pattern in a circuit.

Jackson and homeostasis. Don Jackson, a psychiatrist, founded the Mental Research Institute (MRI) in 1959 and, with Ackerman, the journal Family Process in 1962. His contribution was the application of homeostasis to families: a family is a rule-governed system that maintains itself against change, and symptomatic behaviour can be part of that maintenance. He also introduced **family homeostasis** as a clinical observation: when one member improves, another may worsen, because the system's set point has not changed.

Haley and strategy. Jay Haley's Strategies of Psychotherapy (1963) and Problem-Solving Therapy (1976) treated symptoms as tactics in a power struggle, and therapy as a contest of influence to be won by the therapist's directives. Out of this came **strategic therapy**: prescribing the symptom, ordeals, paradoxical instructions, and a therapist who takes responsibility for change. Haley's later collaboration with Cloe Madanes produced a body of technique that is unfashionable and still effective in the right hands.

Watzlawick and the axioms. Pragmatics of Human Communication (1967) gave the field a vocabulary: one cannot not communicate; content and relationship levels; punctuation; digital and analogic codes; symmetrical and complementary relationships. Two of these are now so embedded in clinical language that students use them without knowing their source, which is exactly what a foundational vocabulary looks like.

What wave 1 got right. The unit is the sequence. Communication is not a skill laid on top of a relationship; it is the relationship. Change the sequence and the symptoms move.

What wave 1 got wrong. It was often manipulative (therapists withholding information, using paradox on unconsenting families), it was blind to power and gender, and its central aetiological claim about schizophrenia was false. Students should be able to say all three without either dismissing the wave or defending it.

3.4 Wave 2: structure and generations (1965 1980)

Wave 2 made the field clinical. It brought families into rooms, built training programmes, and produced the models still most widely taught.

Minuchin and structure. Salvador Minuchin, trained as a paediatrician and psychoanalyst, worked with delinquent boys at the Wiltwyck School and then led the Philadelphia Child Guidance Clinic from 1965. His observation was that these families were not deficient in insight or motivation; they were **organised** in ways that made the child's symptom functional a weak parental subsystem, a child in coalition with one parent against the other, boundaries too diffuse or too rigid. Families and Family Therapy (1974) remains the clearest book in the field. His techniques joining, enactment, unbalancing, restructuring, mapping are the backbone of MFT 706.

Haley in Philadelphia. Haley joined Minuchin in 1967, and the exchange between structure and strategy produced much of what is now standard: the therapist as an active organiser of the session, the deliberate use of directives, the assumption that change in behaviour precedes change in understanding.

Bowen and the generations. Murray Bowen, at the Menninger Clinic and later Georgetown, built the most elaborate theory in the field: **differentiation of self** (the capacity to stay emotionally connected while thinking for oneself), **triangles** (the three-person unit as the molecule of emotional systems), **multigenerational transmission** (patterns passed down and intensified across generations), **family projection** (a child recruited to carry the family's undifferentiation), **emotional cutoff**, and **societal regression**. Bowen's therapy was explicitly about the therapist's own family of origin: trainees were required to work on their own genograms and to re-enter

relationships with their families of origin as the core of their training. That requirement is why this library asks you to do a reflective task every week.

Satir and the person. Virginia Satir brought warmth, communication training, family sculpture and an explicitly humanistic value system. Her categories placator, blamer, computer, distractor, leveller are crude and useful: students recognise them instantly, and so do families. She also insisted, against the cybernetic mainstream, that the therapist's own personhood is the instrument, a claim that the common-factors literature later supported with less charm.

The experiential wing. Carl Whitaker's symbolic-experiential therapy deliberately unsettling, co-therapy with a live team, the therapist's own fantasies used in the room represents a road the field mostly did not take, and its influence survives in the use of self and in the practice of working with the therapist's own reactions in the moment.

> **PRACTICAL**
> Take the Okonkwo family. Write four sentences describing the same session as Minuchin, Bowen, Satir and Whitaker would each have conducted it. Where do they differ: in the seating, the first question, the target of change, or the theory of what a symptom is?

3.5 Wave 3: meaning (1975 1990)

The Milan group Selvini Palazzoli, Boscolo, Cecchin and Prata began as strategic therapists and became something else. Their Paradox and Counterparadox (1978) described a team working behind a one-way mirror, long intervals between sessions, and interventions delivered as letters: rituals, positive connotation, and the **prescription of the symptom**. The style was hierarchical, elegant and, in retrospect, ethically awkward.

By the early 1980s the group had split, and the second phase associated especially with Cecchin turned from intervention to **meaning**. The key concepts: **hypothesising** (a provisional explanation, held lightly and revised), **circularity** (the interviewer's discipline of asking about differences and relationships), **neutrality** (curiosity about all positions rather than indifference), and **circular questioning** as the primary instrument. The therapist's job was less to change the system than to introduce information that the system could not have generated itself.

Second-order cybernetics entered here, through the work of Heinz von Foerster and the writing of Lynn Hoffman ("Beyond power and control", 1985). The observer is part of the observed system; there is no Archimedean point; therapy is a conversation in which the therapist's own descriptions are part of what is described. This is intellectually attractive and clinically dangerous, because a therapist who is only curious can leave a violent household undisturbed. The corrective came from the outside, in wave 5.

The reflecting team. Tom Andersen, in Troms , Norway, developed the reflecting team (1985 87): the team observes and then discusses in front of the family, with the family listening, in a tone of tentative curiosity. The move distributes expertise and gives the family the experience of being thought about rather than judged. It is one of the few techniques of this period that has survived intact and is used across models.

3.6 Wave 4: solutions and stories (1980 2000)

Two movements, both impatient with pathology, both popular with trainees because they feel respectful, and both vulnerable to the criticism that respect can shade into avoidance.

Solution-focused brief therapy. Steve de Shazer, Insoo Kim Berg and colleagues at the Milwaukee Brief Family Therapy Center built an approach around the question "what would be different if the problem were solved?" the **miracle question** plus **scaling**, **exceptions** (times the problem did not happen), and **coping questions**. The stance is minimal-intervention, client-as-expert, and oriented to the next small step. The strength: engagement, brevity, respect. The weakness: when there is trauma, violence or serious mental illness, a focus on solutions can leave the worst of the situation unaddressed, and the therapist may not notice, because the conversation is pleasant.

Narrative therapy. Michael White and David Epston, in Adelaide and Auckland, worked from the premise that people organise experience through stories, and that problems are maintained by **dominant stories** that narrow what a person can be. The techniques: **externalisation** ("the problem is the problem, the person is not the problem"), **unique outcomes** (moments that contradict the dominant story), **re-authoring**, **definitional ceremonies**, and the use of letters and documents. The political commitment is explicit: narrative therapy is designed to notice and resist the power of diagnosis, institutions and cultural norms including, in White's later work, the norms that shape gender and family life.

Collaborative and dialogic practice. Harlene Anderson and Harry Goolishian's collaborative language systems approach, and the open-dialogue work of Jaakko Seikkula in Finnish Western Lapland, pushed further: therapy as a conversation among many voices, the therapist as a participant in a dialogue rather than an agent of change. Open dialogue's outcomes with first-episode psychosis have been widely discussed and are methodologically contested; students should read the primary papers rather than the summaries, which are usually more enthusiastic than the data.

> **DEBATE**
> "Solution-focused and narrative therapy are the most ethical models in the field, and the least adequate for serious risk." Twenty minutes. Both halves of the sentence need defending.

3.7 Wave 5: evidence, attachment and integration (1990 present)

Contemporary practice is not a single wave but a convergence, and students should be able to name its parts.

The evidence movement. Manualised treatments, randomised trials, meta-analysis, and the rise of evidence-based practice as a policy requirement. Its achievements are real: couple therapy for relationship distress, behavioural parent training, family-based treatment for anorexia, multidimensional family therapy for adolescent substance use, and systemic work in paediatric chronic illness all have credible support. Its costs are also real: manualisation can strip the flexibility that made systemic work effective; trials

under-represent the families who actually attend public clinics; and the hierarchy of evidence privileges questions about average effects over questions about mechanism and meaning.

The common-factors turn. Research on the alliance, on therapist effects and on client factors demonstrated that a large share of outcome variance is not attributable to the model. This did not make models irrelevant; it made the fit between model, therapist and family the central question. Feedback-informed treatment the routine use of session-by-session outcome and alliance measures is the practical descendant, and it is now a standard of good practice rather than an innovation.

Attachment. Emotionally focused therapy (Sue Johnson, Les Greenberg) rebuilt couple therapy on attachment theory and emotion science. It is the most influential couple model of the last thirty years, and its evidence base including several randomised trials and meta-analytic support is the strongest of any couple approach. Attachment thinking also reorganised work with children, fostering and adoption (see MFT 711).

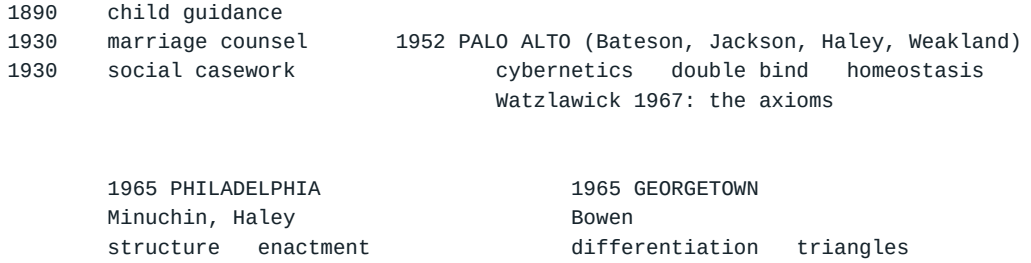
Neuroscience. Findings on the social brain, stress physiology, co-regulation and memory have enriched the field's language and some of its techniques (see MFT 726). The danger is **neuro-pseudo-profundity**: using brain vocabulary as though it explained what it merely redescribes. Polyvagal theory, in particular, is clinically generative and scientifically contested; a professional should be able to use its metaphors while saying honestly that the mechanism is not settled.

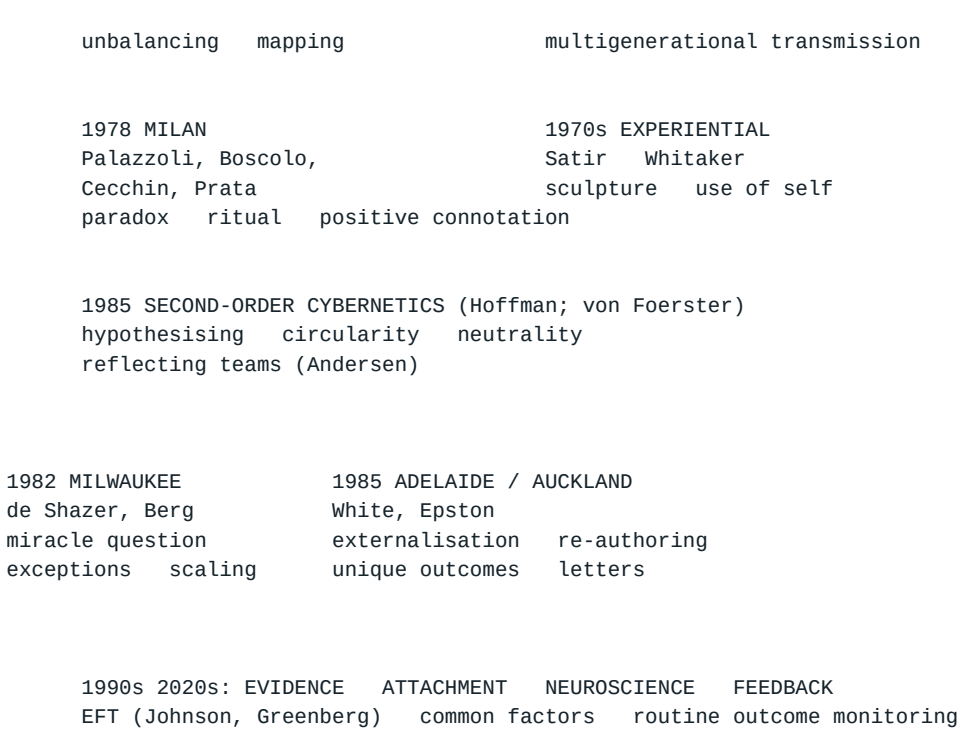
Integration and the common structure. Most practitioners now describe themselves as integrative. The substantive question is not whether to integrate but how: by theory (a superordinate framework), by technique (borrowing tools), by sequence (matching the model to the phase of work), or by feedback (letting outcome data choose). MFT 725 takes this up properly; here it is enough to note that "integrative" is a description of a problem, not a solution.

> **TASK**
> Write 300 words predicting the sixth wave. Defend it with two pieces of evidence from the last five years and one prediction that could be shown to be wrong within a decade. Keep it; you will reread it in Week 14.

3.8 The family tree as a diagram

The history is easier to hold as a picture than as a list. Draw it once, by hand, and you will not need to revise it.





Read the diagram three ways. **Vertically**, as descent: Milan comes out of Palo Alto, solution-focused and narrative come out of Milan's turn to meaning. **Horizontally**, as disagreement: Philadelphia and Georgetown were contemporaries answering the same question differently, and their disagreement about whether the target is structure or differentiation is still unresolved. **Diagonally**, as return: the contemporary interest in attachment is a return to wave 1's question about emotional bonds, with better instruments.

What the diagram leaves out, and why it matters. The feminist critique (Hare-Mustin, Goldner, Walters, Bograd, and later intersectional writers) is drawn as an arrow into the whole picture rather than a branch: it did not produce a school but it changed every school's self-understanding, by insisting that power, gender and violence are not background variables. The same is true of the cultural critique, which this library treats as constitutive rather than supplementary (MFT 708).

3.9 What each wave still gives you

Students often ask, reasonably, which model to learn. The honest answer is that each wave contributes something you will use whatever you call yourself, and that the contributions are not equally large.

Wave	Permanent contribution	What to leave behind
Communication	Sequence, punctuation, feedback vocabulary, the idea that communication is the relationship	Manipulation; treating paradox as a default; the double-bind aetiology
Structure and generations	Enactment, joining, mapping, subsystem and boundary language, the multigenerational lens, work on the therapist's own family	Rigid normative assumptions about "healthy" family form

Wave	Permanent contribution	What to leave behind
Meaning	Hypothesising, circular questioning, reflecting teams, curiosity as a discipline	Neutrality as an excuse for inaction in the face of danger
Solutions and stories	Engagement, brevity, externalisation, respect for the family's own language and values	Avoidance of trauma, risk and diagnosis dressed as respect
Evidence and integration	Outcome measurement, attachment science, feedback-informed practice, training in what demonstrably helps	Manual-worship; neuro-vocabulary substituting for mechanism

The test of a professional. Can you say, for a specific family, why you chose a particular stance and what you would have done differently if you had chosen another? The therapist who cannot answer that question is not eclectic; they are unskilled and lucky.

3.10 The Adeyemi family seen through five lenses

The same family, five ways. This exercise is the heart of the week: it demonstrates that the models are not competing truths but different instruments, each of which reveals something the others hide.

Communication lens (wave 1). The sequence: father criticises mother defends Amara father withdraws Amara refuses school mother appeals to father father criticises. Punctuation: father says "she is out of control because you indulge her"; mother says "he is never here and then he interferes". The intervention would be to slow the sequence, mark the punctuation, and change one link for example, agreeing that criticism happens only in a scheduled conversation. What this lens sees: the mechanics of escalation. What it misses: the father's grief about his job, which is the fuel.

Structural lens (wave 2). The parental subsystem is weak; Amara is in a coalition with her mother against her father; the boundary between mother and daughter is diffuse. The intervention: strengthen the parental subsystem by giving the parents a joint task, move Amara out of the executive position, and re-establish the sibling subsystem by giving the brother a role that is not "the easy one". What this lens sees: hierarchy and coalition, and it produces action immediately. What it misses: the meaning of the father's drinking and the mother's depression, which structure alone treats as noise.

Multigenerational lens (Bowen). The father's own father was a proud man who lost everything in a business failure and drank; the father's withdrawal is a repetition he has never named. The mother is the child who managed her own mother's depression and has recruited Amara to the same role family projection in progress. The intervention: a genogram across three generations, work on the father's relationship with his own father, and coaching the mother to differentiate rather than to manage. What this lens sees: the timeline that explains why this family, now. What it misses: the immediate sequence, and the risk that a multigenerational story becomes an excuse ("it was passed down to us").

Meaning lens (wave 3). What does the family believe is happening? Mother: "I am failing as a mother." Father: "I am no longer the man of this house." Amara: "If I leave, something will happen." Brother: "Nobody notices me." The intervention: circular

questions that make these beliefs audible to one another; a reframe that the family is protecting each other rather than failing; a ritual a weekly family conversation with a fixed structure that lets new meanings be enacted. What this lens sees: the moral world of the family and its capacity for loyalty. What it misses: the practicalities of a household in financial strain.

Solution-focused lens (wave 4). Exceptions: what happens on the days Amara goes to school? (Her mother's sister visited; the father was at home in the morning.) Scaling: where are you today, 0 to 10, on "this family coping"? What would one point higher look like this week? The intervention: build on the exceptions, deliberately, one small step at a time. What this lens sees: competence, and it engages a family exhausted by being a problem. What it misses: the depression and the drinking, which are not solved by scaling.

The point of the exercise. A formulation is a choice of lens for a purpose, and the competent clinician uses at least two lenses and says which they are using. The incompetent clinician uses one lens, calls it "the evidence-based approach", and cannot explain what it hides.

> **PRACTICAL**
> In groups of five, each person takes one lens and prepares a five-minute opening for the family. Present in role. The observers score each opening on two criteria: does it fit the family's own language, and does it make the next step obvious?

3.10a Four primary sources, read closely

History is learned from secondary summaries at the student's peril. Below are four short extracts, each with what to notice. You do not need to agree with any of them; you need to be able to say what they claim and what they assume.

Bateson, Jackson, Haley and Weakland (1956), on the double bind. The authors describe a mother who, when her child approaches, stiffens; when the child withdraws, she asks why he does not love her. The child cannot win and cannot comment on the game. What to notice: the paper is hedged much more carefully than its reputation suggests the authors call it a hypothesis and explicitly disclaim a simple causal claim; and the language of "victim" is used without any account of what the mother herself is experiencing. That absence is the seed of the feminist critique, twenty years early.

Watzlawick, Beavin and Jackson (1967), on punctuation. "The nature of a relationship is contingent upon the punctuation of the communicational sequences between the communicants." What to notice: this is a claim about description, not about causation. The authors do not say both parties are equally responsible; they say each behaves as though the other started it. Students who muddle those two claims produce circularity without accountability, which is the error discussed in Week 2.

Minuchin (1974), on structure. "The therapist joins the family in a position of leadership and then uses himself to change the organisation." What to notice: the therapist is inside, and directive. The sentence is unimaginable in the Milan writing that followed, and it is the ancestor of every modern discussion of the therapist's use of self.

White (1989), on externalisation. "The problem is the problem; the person is not the problem." What to notice: the sentence's power is grammatical. It separates a person from a description, which changes what the family can do; and its weakness is that some problems are also acts violence is not merely a story, and a person who commits it is not only a narrator.

3.11 Annotated transcript: history in one sentence

Students often experience the history as inert because it is taught as a list of names. It comes alive when you see a clinician choose a tradition in a single sentence. Four openings, all defensible, all different.

> **OPENING A (STRUCTURAL):**
> "Before we talk about Amara, I would like to see how the two of you decide something together. Can you choose something small the next school letter, say and decide it now, in front of me?"

What this does. It creates the parental subsystem by asking it to act, and it gives the therapist live data rather than reported data. It comes straight out of Minuchin's enactment. What it costs. It is directive, and a family that feels managed will resist it; it presumes the parents can act together at all.

> **OPENING B (BOWENIAN):**
> "I want to understand where this pattern comes from. Could you tell me about your own parents' marriage what happened in their house when money was short?"

What this does. It places the current difficulty in a multigenerational frame and begins the process of differentiation by asking an adult to think about their family rather than react within it. What it costs. It can feel like a history lesson when the family wants relief, and it can deflect from immediate risk.

> **OPENING C (MILAN):**
> "I am going to ask some unusual questions, and I want each of you to answer for someone else. Amara, who in this family is most worried about your mother? And who is least worried?"

What this does. It builds a map of the system's concern and immediately distributes the problem across relationships. What it costs. It can feel like a parlour game, particularly with a family in acute distress, and it can produce abstract answers that the therapist then over-interprets.

> **OPENING D (SOLUTION-FOCUSED):**
> "Tell me about a day in the last month when things went better than usual even slightly. What was different about that morning?"

What this does. It starts from competence and gives the family an experience of itself as capable, which is powerful with a demoralised household. What it costs. If the honest answer is "there hasn't been one", the therapist is left holding an approach the family has just refuted; a skilled practitioner will have a bridge ready ("then tell me how you have kept going").

The lesson. Each opening is a hypothesis about what will help, expressed as a first move. Your training should leave you able to make any of the four deliberately, and able to explain afterwards why you chose the one you chose. That capacity not loyalty to a school is what "integrative" should mean.

> **DRILL**
> Deliver all four openings to the same peer, in role as the mother. Ask which one made her want to come back, and which one made her want to leave. The answers will surprise you and will differ by person, which is the point.

3.11a How the field's arguments became clinical habits

The history is not over; its disagreements are embedded in the language clinicians use, usually without attribution. Five examples.

"Let's not get into the past" comes from the strategic and solution-focused waves, which treated history as a distraction from the loop. It is right about the futility of archaeology and wrong when the past contains unmourned loss or untreated trauma.

"Everyone in the family is doing their best" comes from Satir and the humanistic wing. It is a useful corrective to pathologising and a dangerous substitute for accountability.

"What would it look like if the problem were solved?" comes from Milwaukee. It is the single most portable question in the field and it will fail with a family for whom the problem is a death.

"That's a boundary issue" comes from Minuchin, filtered through thirty years of training. It is often accurate and almost always unusable as a sentence uttered in the room.

"The system is resisting" comes from the cybernetic wave and is the field's most damaging inheritance. It converts the therapist's failure into the family's pathology. The corrective is a sentence this library repeats: if nothing is changing, check the formulation, the dose, the goal and the fit before you check the family's motivation.

> **AUDIT**
> List five sentences you habitually use in clinical conversation. For each, identify which wave it comes from and what it assumes. Then mark the ones you would be unable to defend if challenged by a colleague, a supervisor or a family member.

3.12 Six errors in using history

1. **The name-drop.** Citing Bateson to a family. Fix: history is for you, not for them; translate every idea into the family's language before it leaves your mouth.
2. **Nostalgia.** Treating the 1970s as a golden age. Fix: name what those clinicians did that would now be unethical, and say why.
3. **Fashion as evidence.** Adopting a model because it is current or because a training was convenient. Fix: ask what evidence supports it for this problem, and what it costs.
4. **Model loyalty as identity.** "I am a structural therapist" as a personality claim. Fix: describe what you do, when, and why; let others attach a label if they need one.
5. **The single-story training.** Learning one model deeply and calling it integration. Fix: the comparative assignment exists for this reason; do it honestly.
6. **Ignoring the critiques.** Teaching the canon without the feminist, anti-racist and survivor critiques. Fix: they are not add-ons; they are the reason the canon changed.

3.12a A supervision case: what happens when the model fails

A trainee presented this in supervision. She had been working with a family of four for six sessions using a structural approach joining, mapping, enacting, unbalancing and nothing had shifted. The parents performed the joint tasks in the room and abandoned them at home. The children were polite and blank. The trainee's formulation, reviewed in supervision, was accurate but incomplete: she had described the family's organisation and had not asked what the organisation was for.

Two questions changed the case.

"What does this family believe would happen if they did the task?" The answer, discovered through circular questions, was that both parents believed that acting together would expose the marriage as a pretence. The father's family had a saying: "a house that discusses its problems is a house with no roof." The mother had been raised to keep the peace at any price, and had watched her own mother absorb her father's rage for thirty years. The structural task was, from inside their world, a request to demolish the house.

"What has this family survived together?" A fire, a business collapse, a stillbirth, and a period of separation the children knew nothing about. This is a family with a history of endurance and a rule that endurance is done silently.

What the trainee did in the next session. She abandoned the enactment task for the moment and used a different instrument: she asked the parents, in front of the children, to describe one thing they were proud of having got through together. It took four minutes and produced more contact between them than six sessions of structure. She then reframed the joint parental task in the family's own terms not "decide together" but "as a family that has survived worse, decide how to handle this one".

The lesson for this week. Models are instruments, and the skill is diagnostic: when an instrument is not working, the question is not "how do I do the technique better?" but "what does the family's world make this technique mean?" That question is historical in its logic it asks which tradition of family life the family itself belongs to, and which of the

field's available languages can reach them. A therapist who has only one model has only one chance of finding a language the family can hear.

3.13 Culture, faith and the Nigerian lineage

The history above is mostly North American and European, and a student in Ibadan should ask what it has to do with them. Two answers.

First, the critiques from the global South are part of the field's history, not a local appendix. The assumption that the nuclear household is the unit, that individuation is the goal, and that the therapist is a neutral observer have all been contested from African, Asian and Latin American contexts for decades. The Nigerian literature in counselling, psychology, social work and pastoral care has its own lineage: community mental health work in the 1970s and 1980s, the integration of traditional healers into referral pathways, pastoral counselling in the churches and alfas and imams in the mosques, and a strong tradition of family-life education. Knowing that lineage is not decoration: it is what allows you to place imported models rather than being placed by them.

Second, the extended family is a theoretical asset, not a complication. Western family therapy spent thirty years discovering that the household is not the system through work on multigenerational process, on non-resident fathers, on kinship care. A clinician trained in a context where grandmothers, aunts, uncles and cousins routinely co-parent starts with that insight rather than arriving at it. The task is to make it explicit and teachable, not to apologise for it.

A caution. Cultural specificity can become a way of exempting families from scrutiny. "In our culture, discipline means beating" is a claim that requires examination, not deference the evidence on corporal punishment is clear, and the child's safety is not a Western import. The discipline is to take the family's frame seriously and to hold the professional's obligations, including the legal ones (see MFT 724).

> **REFLECT**
> Which of the five waves most resembles how your own family handles difficulty? Which one most resembles how you were taught to handle it professionally? Write 250 words on the gap between them.

3.13a The Nigerian and African lineage, in more detail

A student who learns only the North American canon will arrive at a professional identity that is borrowed. The local lineage is substantial and should be known.

Community psychiatry and the extension of care, 1960s 1980s. The Aro Mental Hospital in Abeokuta, and its community-based programme from the early 1960s under T. Adeoye Lambo, is a landmark in world psychiatry: it replaced custodial care with village-based care, involved families and traditional healers, and demonstrated outcomes that were, for their time, remarkable. Family involvement was not an added technique; it was the model. Any honest history of systemic practice has to include it.

Traditional and faith-based healing as part of the referral system. Herbalists, babalawos, pastors, alfas and imams are, for most Nigerian families, the first port of call and sometimes the only one. The professional task is not to compete but to understand, coordinate and where a practice is dangerous to say so clearly while keeping the family engaged. Research on collaboration between traditional healers and formal services has a long history in Nigeria and West Africa, and the practical lessons are consistent: respect, clarity about what each can offer, and an explicit safety floor.

Family life education and pastoral counselling. Churches and mosques run premarital counselling, marriage enrichment, and considerable informal couple and family work. Many of the referrals you will receive come through these channels, and many of the families you see will have had a pastoral conversation before they had a clinical one. Knowing what those conversations contain and their limits makes you a better collaborator and a less anxious professional.

The academic lineage. Departments of psychology, sociology, guidance and counselling, and social work in Nigerian universities have produced a substantial literature on marriage, family structure, child-rearing, migration and mental health. Students writing dissertations (MFT 721, MFT 770) should search it deliberately: the international databases are not neutral archives, and a literature review that cites only Northern journals will have missed the studies with the most relevant samples.

What to take from this. The relational frame is not an import that must be adapted to local reality; it is an insight that local practice arrived at independently, with different vocabulary and different institutional forms. Your task is to learn both lineages and to be able to say, for any particular technique, whether it travels, what it needs to travel, and what it might destroy on arrival.

3.14 The comparative essay: how to write it

Half of the marks for this course come from a comparative essay of 2,000 words on two models applied to one case. Students lose marks in predictable ways, and the essay is easier than it looks if you follow the structure.

The brief. Choose two models from different waves for example, structural (wave 2) and narrative (wave 4), or Milan systemic (wave 3) and emotionally focused (wave 5). Choose one family from the case bank. Write an essay with the following sections, in this order.

1. **The presenting pattern** (250 words). The family as they arrive: who is worried, who is present, what has been tried, what the referral says. No theory yet. This section establishes that you can describe before you interpret a skill examiners find conspicuously rare.
2. **Model one: how it sees** (450 words). The model's assumptions: what a symptom is, where pathology lives, what counts as the unit, what counts as change. Then the formulation in that model's terms, in its vocabulary, with the specific evidence from the case that supports it.

3. **Model one: what it would do** (250 words). Two or three concrete moves, in sequence, with the reasoning. Not "use enactment"; rather, "ask the parents to decide the school letter in front of Amara, because the parental subsystem is the target and doing it live gives me better data than asking about it".
4. **Model two: how it sees** (450 words). As above. Resist the temptation to make model two a foil; a comparison in which one model is obviously superior is a failed comparison.
5. **Model two: what it would do** (250 words). As above.
6. **The comparison** (250 words). Where do the two formulations conflict? Where do they merely differ in emphasis? What does each model see that the other cannot? Which would you choose for this family, and on what grounds evidence, fit, the family's preferences, your own competence, or the setting's constraints? The grounds must be explicit.
7. **Conclusion** (100 words). What you have learned about the relationship between theory and practice.

The six ways students lose marks. Describing models without applying them to the case; applying both models to different aspects of the case so no comparison occurs; declaring one model "more holistic" without evidence; using the models' jargon without translating it; ignoring power, culture and risk; and failing to state a reasoned preference.

Model paragraph, for calibration. Here is a body paragraph at distinction level.

> Structural family therapy locates the problem in the organisation of the family rather than in the individual. Minuchin's claim is that symptomatic children are frequently occupying a position in the hierarchy that belongs to adults in coalition with one parent against the other, or parentified by an absent adult. This is visible in the Adeyemi family in three places: Amara sits beside her mother and answers questions addressed to her; she describes managing her mother's low mood; and her father, when he attends, is treated by both as a visitor whose opinion arrives from outside the household. The structural formulation is therefore that the parental subsystem is not functioning as a subsystem, and that Amara's school refusal is a stabilising move within that structure. What this formulation gains is a target that can be acted on in the room: the therapist can create the parental subsystem by giving the parents a joint task and observing them attempt it. What it risks losing is the meaning of the father's absence to the father his own redundancy, his own father's drinking which structural language can describe as "distance" but cannot explain.

Note what the paragraph does: it names the model's claim, applies it to specific observable evidence, states what the formulation buys and what it costs. That last sentence is what separates a distinction from a competent pass.

3.15 Timeline for revision

Twenty dates, in the order a student should be able to produce them. The examination sometimes asks for a timeline, and always rewards precision about which decade a development belongs to.

Year	Event	Why it matters
1942	American Association of Marriage Counsellors founded	The couple as a clinical unit, before "family therapy" existed
1952	Bateson's group begins work at Palo Alto	Cybernetics enters the study of families
1956	"Toward a theory of schizophrenia" published	The double bind; the field's founding controversy
1959	Mental Research Institute founded (Jackson)	An institutional base for communication research
1960	Ackerman Family Institute, New York	Clinical training in family therapy begins
1962	Family Process founded (Ackerman and Jackson)	The field acquires a journal and a canon
1963	Haley, Strategies of Psychotherapy	Strategic therapy formalised
1965	Minuchin leads the Philadelphia Child Guidance Clinic	Structural family therapy develops
1966	Bowen, "The use of family theory in clinical practice"	Differentiation and triangles published
1967	Watzlawick, Beavin and Jackson, Pragmatics	The axioms; the field's vocabulary fixed
1972	Bateson, Steps to an Ecology of Mind	The cybernetic essays collected
1974	Minuchin, Families and Family Therapy	The clearest clinical text in the field
1978	Selvini Palazzoli et al., Paradox and Counterparadox	Milan systemic therapy; the team behind the mirror
1980	"Hypothesizing circularity neutrality" published	The technique of circular questioning formalised
1982	Milwaukee Brief Family Therapy Center founded	Solution-focused work begins
1985	Hoffman, "Beyond power and control"	Second-order cybernetics enters clinical writing
1985 87	Andersen's reflecting team papers	The team discusses in front of the family
1990	White and Epston, Narrative Means to Therapeutic Ends	Narrative therapy consolidated
1990s	EFT trials begin to appear (Johnson, Greenberg)	Attachment-based couple therapy acquires evidence
2000s	Feedback-informed treatment; routine outcome monitoring	The model matters less than the fit and the feedback

3.16 Self-test

Multiple choice, then short answers.

1. The double-bind hypothesis was developed by: (a) Minuchin and Haley (b) Bateson, Jackson, Haley and Weakland (c) Bowen and Kerr (d) White and Epston
2. "The problem is the problem; the person is not the problem" is a principle of: (a) Structural therapy (b) Strategic therapy (c) Narrative therapy (d) Bowenian therapy

3. Which model is most associated with enactment and unbalancing? (a) Milan systemic (b) Structural (c) Solution-focused (d) Collaborative
4. Positive connotation and ritual are techniques of: (a) The Milan group (b) The Milwaukee group (c) The Philadelphia group (d) The Galveston group
5. Differentiation of self, triangles and multigenerational transmission belong to: (a) Satir (b) Whitaker (c) Bowen (d) de Shazer
6. Second-order cybernetics holds that: (a) Systems have two levels (b) The observer is part of the observed system (c) Feedback operates twice (d) Two therapists should work together
7. The reflecting team was developed by: (a) Andersen (b) Anderson and Goolishian (c) Seikkula (d) Cecchin
8. Which is the strongest evidence-supported couple therapy? (a) Milan systemic (b) Emotionally focused therapy (c) Strategic therapy (d) Collaborative therapy

Answers. 1 b 2 c 3 b 4 a 5 c 6 b 7 a 8 b.

Short answers.

What did wave 3 add that wave 1 lacked, and what did it lose? A strong answer names meaning, hypothesis, circular questioning and the reflecting team; then names the loss therapeutic ambition, responsibility for change, and attention to power and risk.

Why is "integrative" a description of a problem rather than a solution? A strong answer distinguishes integration by theory, technique, sequence and feedback, and shows that each requires a criterion for choosing; without a criterion, integration is incoherence with good manners.

3.16a What the examiners look for

A note on assessment for this week's material, since the examination question on history is one of the most predictable and the most frequently answered badly.

Three habits earn marks. Precision about dates and decades (not "in the seventies" when you mean 1978); attribution that is correct (positive connotation is Milan, not Minuchin); and evaluation what each wave gained and lost, stated in the same paragraph as the description.

Three habits lose them. The list, in which each model is described in a sentence and nothing is compared; the hagiography, in which founders are praised rather than understood; and the anachronism, in which 1950s claims are judged by 2020s standards without noting that the standards themselves are a product of the intervening argument.

A model sentence for evaluation. "Milan's turn to meaning gave the field its most refined interview technique and deprived it, for a decade, of a theory of action; the reflecting team is best understood as an attempt to recover action without abandoning curiosity." A sentence like that demonstrates that you can hold a gain and a loss at once, which is the intellectual skill this course is trying to build.

3.17 Reflective task

Write 300 words on the model you find most attractive and the model you find most repellent. For each, name the reason and then check the reason against your own family history. Students usually discover that the attractive model is the one that would have helped their family and the repellent one is the one their family was subjected to. That discovery is not a debunking; it is professional formation, and it is the reason supervision exists.

3.18 Readings for this week

- **Nichols & Schwartz**, Family Therapy: Concepts and Methods, chapters 1 4. The best overview; read it as a map, not as a verdict.
- **Hoffman (1981)**, Foundations of Family Therapy. The history written by someone who was there.
- **Minuchin (1974)**, Families and Family Therapy, chapters 1 3. Read for the clarity of the clinical reasoning, not for the model.
- **Bowen (1978)**, Family Therapy in Clinical Practice, chapters 1 5. Dense, idiosyncratic, foundational.
- **White & Epston (1990)**, Narrative Means to Therapeutic Ends, chapters 1 2. Externalisation, in the original.
- **de Shazer (1985)**, Keys to Solution in Brief Therapy. The miracle question and its rationale.
- **Hare-Mustin (1978)** and **Goldner (1985)**, as for Week 2. The critiques that changed the canon.

Next: Week 4 Systems concepts you will actually use

3.16b Two worked formulations, same family, different waves

The clearest test of historical knowledge is the ability to formulate the same family in two vocabularies without mixing them. Here are two, both about the Adeyemis, both defensible, written to be read side by side.

Structural formulation. The household is organised around a weak parental subsystem. The father is peripherally involved: he is present, but he occupies a visiting position, offering opinions from outside the executive of the family. The mother daughter boundary is diffuse; Amara functions as her mother's emotional companion and, in the mornings, as the person who absorbs and then repairs her mother's distress. A stable cross-generational coalition has formed. The school refusal is not a symptom that arrived from nowhere; it is the behaviour that keeps the coalition intact by keeping Amara at home. Target of change: strengthen the parental subsystem, move Amara out of the executive position, and re-establish the sibling subsystem so that the younger brother is not recruited into the space Amara vacates. First move: a joint parental task, attempted in the session, on a real decision.

Bowenian formulation. The family's level of differentiation is low in the marital dyad, and the anxiety that this generates has been triangulated through Amara, who is the child most attuned to her mother and therefore the most available for the family projection process. The father's emotional cutoff from his own family of origin he has not spoken to his brother in nine years, and his father drank has left him with no vocabulary for the shame of redundancy and no relationship in which to think about it. The mother's over-functioning is a repetition of her position in her family of origin, where she managed her own mother's depression. Target of change: increase differentiation the capacity of each adult to stay connected while thinking for themselves and reduce the triangulation of Amara. First move: a three-generation genogram, followed by coaching the mother to define a position to her own mother, and the father to re-open contact with his brother.

What to notice in comparing them. Structural language gives you an immediate, testable, in-session action; Bowenian language gives you a timeline and a theory of why these particular people, and it produces change slowly, through the adults' own development. Neither is a description of the family; both are descriptions of what the clinician has decided to notice. The mark of a trained clinician is that the choice between them is explicit and defensible rather than temperamental.

> **TASK**
> Write a third formulation, 300 words, in the vocabulary of narrative therapy. Then write one sentence stating what your narrative formulation can see that neither of the above can, and one sentence stating what it cannot see.

Week 4 Systems concepts you will actually use

4.1 The lecture in ninety minutes

Weeks 1 to 3 built the frame. This week supplies the vocabulary you will use in every session for the rest of your career. It is a technical week, and students often find it the most useful of the first month, because for the first time they can describe what they are seeing rather than merely reacting to it.

The organising principle: a concept earns its place only if it changes what you do. Every term in this week is therefore taught with three things attached what you can observe that tells you it is present, what it costs the family, and what you would do about it.

Minutes	Segment	What the lecturer does	What students do
0 10	The test of a concept	Two formulations, one with and one without usable concepts	Decide which they could act on
10 25	Boundaries and subsystems	Live mapping with a volunteer family of dolls	Map a family from a transcript
25 40	Triangles, coalitions, detouring	The three-person diagram, drawn and then applied	Identify the triangle in a case

Minutes	Segment	What the lecturer does	What students do
40 55	Rules, roles, myths, secrets	The invisible rule exercise	Write three rules from a family description
55 70	Communication concepts	Metacommunication, levels, symmetry and complementarity	Mark levels in a four-line transcript
70 85	Mapping the Adeyemis	Draw the map, defend every symbol	Draw and defend in pairs
85 90	Set the lab	The structural map assessment	Prepare a map for next week

4.2 What makes a concept usable

A concept is usable when it meets four criteria.

1. **Observable.** You can point to something in the session that would count as evidence. "Enmeshment" fails this test in most students' hands; "mother answers questions addressed to her daughter, three times in ten minutes" passes it.
2. **Relational.** It describes something between people, not inside one person. "Triangulation" passes; "manipulative" fails.
3. **Actionable.** It tells you where to intervene, or at least where to look next.
4. **Falsifiable.** You can be wrong. If no observation would make you withdraw the description, you are not describing the family; you are describing your own frame.

The discipline of this week is to translate every concept into observables before you use it in a report, a supervision or a session. It is the difference between a clinician and someone who has read about clinicians.

4.3 Boundaries

A **boundary** is the set of rules that governs who participates in what, and how close members may be. It is not a wall; it is a permeable membrane that regulates contact, and it exists between subsystems (parent/child, couple/siblings), between the family and the outside (school, kin, services), and inside individuals (self/other, thought/feeling).

Minuchin's three-way classification is still the most useful, provided you replace his normative tone with observational criteria.

Type	What you observe	Typical cost	Typical presentation
Clear	Adults decide adult matters; children are informed appropriately; each member has privacy and a voice	None inherent; flexibility is the point	Families who manage difficulty without losing function
Diffuse	Members answer for each other; privacy is treated as secrecy; distress in one member is felt by all and acted on by all	Chronic anxiety; difficulty individuating; a child recruited as confidant	"We are very close" families in which one adolescent cannot leave

Type	What you observe	Typical cost	Typical presentation
Rigid	Information is compartmentalised; emotion is not discussed; contact is formal or distant	Isolation; secrets; children who cannot read emotion	"We don't have problems" families who present late and in crisis

Observational markers of a diffuse boundary. One member finishes another's sentence; a parent answers a question put to a child; the family reports that everyone knows everything; a member's mood is described as the household's weather; there is no closed door in the house; alliances are assumed rather than chosen.

Observational markers of a rigid boundary. The family reports that a member "has never been one to talk about feelings"; there is a topic nobody names; the parents describe the marriage as "not the children's business" and also cannot discuss it with each other; siblings are estranged; contact with extended family has been cut after a dispute.

Two cautions. First, boundaries are culturally specified. A household in which three generations share sleeping space, income and child-rearing is not enmeshed by definition; the question is whether its members can think, disagree and move. Second, boundaries fluctuate with circumstance: after a death, a migration or an illness, a family will temporarily close ranks, and that is adaptive. Diagnose a boundary only when it is stable across contexts and costly to someone.

> **PRACTICAL**
> Listen to five minutes of any family conversation (a recorded role-play is fine). For each member, count: (a) times they answered for another person, (b) times they were interrupted, (c) times they were asked directly and (d) times they spoke without being asked. The four counts give you a boundary profile in numbers, which is worth more than an adjective.

4.4 Subsystems and hierarchy

Families are organised into **subsystems**: the couple, the parents, the siblings, each individual, and the extended kin who function as a subsystem even at a distance. Each subsystem has its own rules, its own language and its own legitimate authority.

Hierarchy is the arrangement of authority and responsibility. In families with children, the parental subsystem carries executive authority: decisions about money, schooling, discipline, safety and contact with the outside world. That authority is not a value judgement about who matters; it is a functional necessity, because someone has to decide, and children cannot.

Three common structural problems, with their observable signs.

Weak parental hierarchy. The child decides, negotiates or vetoes; parents ask the child's permission; a child is the family's spokesperson; discipline is inconsistent because it is negotiated. Cost: the child carries authority without the capacity for it, which produces anxiety, not confidence.

Cross-generational coalition. One parent and one child form a stable alliance against the other parent. Signs: the child knows about the marriage; the child takes one parent's side in arguments; the other parent is treated as an outsider in discipline. Cost: the child loses the parent they are allied with as a parent the relationship becomes a partnership, and the child's own development is mortgaged.

Parentification. A child takes on practical or emotional care of a parent or siblings. Signs: the child cooks, organises medication, translates, manages the household's feelings. Cost: precocious competence alongside suppressed need; these children present in adolescence or adulthood with anxiety, resentment or a collapse when they finally cannot carry it. Note carefully: parentification is not always harmful. Many children in many households take real responsibility and thrive. The harm correlates with the absence of recognition and support, not with the task itself a distinction that matters in cultures where children's contribution is normative.

4.5 Triangles, coalitions, detouring: three different things

Students conflate these constantly, and the conflation produces bad formulations. They are not synonyms.

A triangle (Bowen) is a three-person emotional configuration that is the smallest stable relationship system. Two people under tension recruit a third to manage it; the third may be a person, an activity, a substance or a symptom. Triangles are normal and universal; the question is how rigid and how costly they are. Observable signs: conversation about a third person whenever two are together; a member who is always discussed and never present; a symptom that appears when the couple's tension rises.

A coalition (Minuchin) is an alliance of two against a third that crosses generational lines. It is a structural fact, not an emotional process: parent and child against the other parent. Observable signs: seating; who looks at whom when a topic is raised; who defends whom; who is described as "the one who understands".

Detouring is a specific manoeuvre: the couple's conflict is displaced onto a third party usually a child so that the couple never has to fight. Two forms: **detouring-attacking** (the child is defined as the problem and both parents unite against them) and **detouring-supportive** (the child is defined as fragile and both parents unite to protect them). Observable signs: the couple's disagreements reliably become conversations about the child; the child's symptoms appear at moments of marital tension; the parents agree warmly about the child and coolly about everything else.

Why the distinction matters. A triangle is a process you interrupt with questions; a coalition is a structure you change with tasks and seating; a detour is a function you name, carefully, when the couple is ready to hear it. The same family can have all three, and the sequence in which you address them is a clinical judgement, not a doctrine.

> **DEBATE**
> "Detouring is a useful concept and a dangerous accusation." Fifteen minutes. Note that the danger is greatest when the child is present: any formulation the family hears as "your child's illness is your marriage" will mobilise defences and may harm the child's relationship with the therapist.

4.5a A triangle worked, carefully

Take a triangle that is present in the Adeyemi household and show how to establish it without asserting it.

The evidence. In the first session, four things happened. The mother described the father's drinking and then, within the same breath, described Amara's sensitivity: "she takes everything to heart, she is like me." When the therapist asked Amara a question about school, the mother answered first, twice. When the therapist asked about the marriage, the mother returned to Amara within one sentence. And when Amara was asked what she worried about, she said "that my mother will be alone".

The inference. The marital tension is being managed through the mother daughter relationship. That is a triangle in Bowen's sense a third party recruited to absorb what the dyad cannot hold and it is also, at the structural level, a coalition, because the alliance is stable and the father is positioned outside it. Note that the two descriptions are of the same phenomenon seen with different instruments.

What you say in the room. Not "Amara is triangulated". The usable sentences are about behaviour and are addressed to the person who can act:

> "Mrs Adeyemi, I notice that when I ask about you and your husband, you tell me about Amara. I may be wrong, but I wonder whether Amara is the person you talk to about the hard things. Is that right?"

Then, to the daughter, without requiring her to betray anyone:

> "Amara, when your mother is upset, what do you do? And what happens to your own evening?"

What you do not do. You do not interpret the triangle to the child, you do not describe the father as "peripheral" in front of his family, and you do not invite the daughter to stop being close to her mother. Each of those moves would produce loyalty conflict in the person with least power, which is the clinical definition of harm in family work.

How you know it is working. Three signs, in order of appearance: the mother begins to answer questions about her marriage with information about her marriage; the daughter volunteers an opinion that differs from her mother's without checking her face; and the father, when present, is addressed by his wife before the daughter is consulted. None of these is dramatic. All of them are structural.

4.6 Rules, roles, myths and secrets

Four concepts that describe the invisible organisation of a family. Each is taught badly in most textbooks, because each is presented as a noun rather than as a procedure for noticing.

Rules. Families run on rules that nobody wrote down: who speaks first, who is consulted, who may be angry, what may be discussed at dinner, who apologises, how money is decided, what happens when someone cries. Most rules are functional and invisible, which is what makes them rules rather than decisions.

Metarules govern the rules: for example "we do not discuss the rules". A family whose metarule is "conflict must not be named" will produce a session in which nothing is argued and everything is tense, and it will describe itself as close.

How to surface a rule. Ask about a specific, recent, ordinary occasion: "What happens at dinner if someone is upset?" "Who decides when the television goes off?" "What would happen if you disagreed with your father in front of your mother?" The answers are rules, stated as customs and customs can be examined.

Roles. Functional roles (the responsible one, the clown, the sick one, the peacemaker, the scapegoat) are distributed early and stabilise the system. The clinical significance is that a role can outlive its usefulness and imprison the person in it: the peacemaker who cannot express anger, the sick one who cannot be well without destabilising the system, the responsible one who cannot fail.

A crucial caution: roles are descriptions of positions in a system, not of personalities. A child in the scapegoat position is not a bad child. When a therapist slips into "he is the difficult one", the formulation has failed.

Myths. A family myth is a shared belief about the family that organises behaviour and resists disconfirming evidence: "we are a happy family", "we never needed anyone", "the men in this family are hot-tempered", "nobody in this family gets depressed, we just get on with it". Myths are not lies; they are the family's constitution, and they are usually protective. They become a problem when reality contradicts them and nobody is allowed to say so.

How to work with a myth. Never by confrontation. The usable route is through an exception or a cost: "You have described a family that never asks for help. What has that cost you?" Or, with a myth of invulnerability: "Which of you would find it hardest to say 'I am not coping'?" The myth is then available to be revised without anybody being humiliated.

Secrets. Not all secrets are damaging; some are protective and some are nobody else's business. Three categories are clinically important. Secrets about the past (a death, a prison term, an adoption, a previous marriage) that are known to some and not others, and that distort the family's explanatory map. Secrets inside the session (an affair disclosed by one partner, a plan to leave, a diagnosis withheld) that place the therapist in an impossible position; the ethical handling of these is treated in MFT 704 and revisited in

MFT 710. Secrets that protect harm (abuse, violence) which must be addressed under safeguarding protocols, whatever the family's wish.

The rule of thumb for secrets. Ask: who is protected by this secret, and who is protected from it? If the answer is the same person, the secret may be kind. If the person protected and the person kept in the dark are different, the secret is doing something else.

> **CASE**
> A 14-year-old is brought for "lying". The parents discovered she had been meeting a boy. In the second session the mother mentions, in passing, that the girl's father is not her biological father and that she does not know. The family myth is "we are completely honest with each other". The therapist's task is not to expose the myth but to establish that the girl's lying is a rational response to a household in which two people hold information the third is not allowed to have and to work with the parents about whether, when and how the disclosure should happen, with the girl's interests, not the therapist's candour, as the criterion.

4.7 Communication concepts worth keeping

Week 2 gave you punctuation and feedback. Four more concepts complete the vocabulary.

Metacommunication. Communication about communication: "I don't mind you being late, it's that you don't text." Metacommunication is how relationships are negotiated, and families vary enormously in how much of it they do. A family with no metacommunication has no way to revise its rules except through crisis.

Levels: content and relationship. Every message carries information and a definition of the relationship. "How was your day?" can be an inquiry or a check on whereabouts. Most couple conflict is a relationship-level dispute conducted at content level an argument about the dishes that is about respect.

Symmetrical and complementary. In symmetrical exchanges, both parties behave as equals (which can escalate into competition). In complementary exchanges, the behaviours are different and mutually fitting (parent/child, pursuer/withdrawer, teacher/student). Neither is healthy or unhealthy in itself; problems arise from rigidity. A couple stuck in complementary pursuit withdrawal needs to build symmetry; a couple stuck in symmetrical escalation needs to accept asymmetry in some domains.

Double-level messages. Messages in which the content and the relationship level contradict each other ("Of course I want you to go out with your friends" said in a tone that forbids it). This is not the double bind the double bind requires that the recipient cannot leave the field and cannot comment on the contradiction. Ordinary double-level messages are common, survivable and worth naming in session.

4.8 The genogram as a conceptual instrument

A **genogram** is a three-generation diagram of family structure, relationships and events. Students often treat it as a data-collection form. It is better understood as a hypothesis machine: its value is in what it makes visible, and in the family's reaction to seeing it.

For this week, three uses only. (Week 8 covers the technique in full.)

1. **To reveal structure.** Deaths, divorces, remarriages, migrations, births, adoptions drawn to scale, across generations make patterns visible that no interview would have produced.
2. **To reveal repetition.** "Your father left at 9, you left at 9, your son is 9." Nobody had said this sentence in the family before.
3. **To reveal the family's own version.** Ask the family to draw it, or to correct yours. The corrections are the data: who is included, who is omitted, which relationships are drawn close and which distant.

Symbols you must know. Male square, female circle, deceased crossed, marriage a horizontal line with the date, divorce two slashes, children descending in birth order, household membership a dotted rectangle, close relationship a double line, conflict a zigzag, cutoff a slash through the line. Learn to draw them small and neat; a genogram drawn badly is a message to the family about how carefully they are being taken.

4.9 The Adeyemi family, mapped

The map, described in words so that you can draw it.

Household. Father (48), mother (45), Amara (16), brother (11). A dotted rectangle around all four with the brother's position marked and a note that he is "the easy one". The paternal grandmother lives 200 km away and is telephoned every day by the mother; draw a close line to the mother, a conflicted line to the father.

Relationships. Between the parents: conflict (zigzag), with a note "arguments about money and the father's drinking". Father to Amara: distant with moments of criticism; draw a dashed line and mark it "unpredictable". Mother to Amara: very close (double line) with an arrow from Amara to the mother marked "caretaking". Mother to brother: close but described as "neglected" by the mother herself. Brother to Amara: close, protective.

Events. Father's redundancy fourteen months ago (mark it on the timeline). Mother's first depressive episode six years ago, and the current one. The father's own father died of a stroke when the father was 19; mark deceased and note that the father "has never talked about it". A stillbirth between the two living children, never discussed.

What the map makes visible that the interview did not. Four things. The father's father died when he was Amara's age, which reframes his difficulty with her adolescence. The stillbirth sits between the two living children and is never mentioned, which suggests an unmourned loss in the mother's story. The grandmother is a daily presence and a structural authority. And the brother is described by everyone as fine, which in a family like this is a hypothesis to test rather than a fact to accept.

How to use the map in session. Show it, ask for corrections, and then ask the two questions the map makes possible: "Who is missing from this picture?" and "What would change on this drawing if therapy worked?"

PRACTICAL

Draw the Adeyemi genogram yourself, from the description above, without looking at the printed version in the appendix. Then compare. Most students omit the stillbirth, place the grandmother wrongly, or fail to mark the father's age at his own father's death which is precisely the kind of omission that makes a formulation respectable and useless.

4.10 Annotated transcript: mapping in the room

T1 Therapist: I am going to draw your family on this paper while we talk. Tell me if I get it wrong.
T2 Mother: (laughs) You will get it wrong.
T3 Therapist: Good. Start me with your husband's family. Who were his parents?
T4 Mother: His father is dead. His mother is alive, she is in Ibadan. He does not talk to her much.
T5 Therapist: (draws) How long has he not talked to her much?
T6 Mother: Since his father died. That was a long time ago. He was still a boy.
T7 Therapist: How old was he?
T8 Mother: (pause) About nineteen. He had to leave school.
T9 Therapist: And Amara is how old?
T10 Mother: Sixteen.
T11 Therapist: (draws, then stops) Is there anything else on this paper that I should know about?

Annotations. T1 announces the task and immediately hands the family the right to correct the therapist. This is a structural move: it makes the map a shared object rather than the therapist's assessment. T2 is a test of the therapist's response to teasing; the answer warmth and an invitation establishes that the room can be informal.

T3 starts with the father's family, who are absent. This is deliberate: it prevents the session from becoming a conversation about the mother, and it surfaces the paternal cutoffs that no one has mentioned. Students almost always start with the presenting child.

T5 T8 produce the most important sentence in the session: the father was 19 when his own father died and he had to leave school. The therapist asks for the age and does not comment further. T9 is the juxtaposition "and Amara is how old?" which lets the mother make the connection herself. She does, in the pause before T10.

T11 is the invitation that produces secrets. "Is there anything else on this paper that I should know about?" is open, bounded by the drawing, and not threatening: it asks about the paper, not about the family's morality.

What the therapist did not do. She did not say "so your husband is repeating his own adolescence with his daughter" which would have been an interpretation the mother could neither confirm nor use. She did not ask about the stillbirth directly, which would have been premature in a first mapping session. She did not draw conclusions from the map in front of the family; the map's job in session one is to gather and to invite.

4.10a Second transcript: naming the rule, not the person

> **T1 Father:** In this house we don't discuss feelings. That is not how I was raised.
> **T2 Therapist:** So the rule is that feelings are handled privately. Who taught you that rule?
> **T3 Father:** My father. He never said a word about anything. He just worked.
> **T4 Therapist:** And what did that give you?
> **T5 Father:** (long pause) It gave me a strong house. Nobody went hungry.
> **T6 Therapist:** And what did it cost?
> **T7 Father:** (longer pause) I don't know what my son is thinking. I don't know what my wife is thinking.
> **T8 Therapist:** Amara, in this family, if you were having a bad week, who would know?

Annotations. T2 is the whole technique of this week in one move: the father has stated a rule, and the therapist treats it as a rule a thing with an author, a history and a cost rather than as a fact about the family or a failing of the father. T4 asks for the gain before the cost, which is why T5 is not defensive; the therapist has protected the father's inheritance before examining it.

T6 is the pivot, and it only works because T4 came first. A therapist who leads with "what did it cost?" produces a defended father; a therapist who establishes what the rule achieved produces a father who can name its limits himself. T7 is the father doing the formulation.

T8 moves the question from the father, who has just disclosed something painful, to the daughter, in a way that lets her answer without contradicting him. Her answer "nobody, unless my mother notices" completes the picture: the rule is not just the father's; it is the household's, and the mother has been carrying the emotional work the rule leaves undone.

4.11 Laboratory: the structural map

The assessed practical. Groups of three. Each group receives a one-page case summary (case bank, appendix) and must produce a structural map, a genogram and a two-minute presentation.

Rules for the map. Every symbol must be justifiable from the text or from an inference you state aloud. Adjectives are not permitted on the drawing; only behaviours and events. Each map must mark at least one hypothesis with a question mark, so that the group declares what it does not know.

Marking rubric.

Criterion	Weight	Full marks
Genogram accuracy	20%	Three generations, correct symbols, dates and deaths, household marked
Structural reading	30%	Subsystems, boundaries and hierarchy named with observational evidence, not adjectives
Triangle/coalition/detour distinction	20%	The correct concept is applied, and the group can say why not the others

Criterion	Weight	Full marks
Hypothesis discipline	20%	At least two competing hypotheses, each with the question that would test it
Presentation	10%	Two minutes, no jargon the family could not follow

4.12 Six errors in using systems concepts

1. **Concept as insult.** "She is enmeshed" said about a mother who is doing what her culture and her circumstances require. Fix: describe the behaviour and its cost before you use any label.
2. **The static map.** Drawing a family once and treating the drawing as the family. Fix: redraw after every third session; families change, and so should your map.
3. **Triangle-spotting as a game.** Finding triangles everywhere, including where the evidence is a single sentence. Fix: a triangle requires evidence of a process a third party recruited when tension rises not merely three people in a room.
4. **Detour accusations.** Telling parents their child's illness is about their marriage. Fix: name the function only when the couple has enough containment to hear it, and never with the child present.
5. **Normative hierarchy.** Assuming that a "proper" family has a particular shape, and pathologising households that organise differently. Fix: ask what the organisation costs and whom, rather than whether it matches a template.
6. **The unmarked inference.** Presenting an inference as an observation, in a report or in a session. Fix: distinguish, out loud, between what you saw and what you think "I noticed your husband answered for you twice; I may be wrong, but I wondered whether that happens often."

4.12a Writing a structural formulation for a report

Students can formulate in the room and freeze at the keyboard. A short template, with the rule that every sentence must survive a challenge from a colleague who has never met the family.

> Structure. The household consists of four members (father 48, mother 45, daughter 16, son 11) with a peripheral but present paternal role and a telephone-based involvement of the paternal grandmother. Hierarchy. Executive decisions are made by the mother; the father is consulted after the event, and his disagreement is registered as criticism rather than as participation. Boundaries. The mother daughter relationship is highly involved and carries emotional confidences; the marriage is comparatively disengaged; the son is described by all members as "easy" and appears to be outside the organising conflict. Hypothesis. The daughter's school refusal is maintained by a sequence in which her presence at home reduces the mother's isolation and defers marital discussion. This is a hypothesis, not a conclusion: it is supported by three observations and would be disconfirmed if the refusal persisted during a period in which the parents' contact increased and the mother's support from others improved. Risk. No current indicators of violence or self-harm were disclosed on direct enquiry; the father's alcohol use was reported at 4 6 units on 5 nights per week and has been raised with him for assessment. Plan. Conjoint sessions including the father; a joint parental task in session; a staged school return negotiated with the school; support for the mother's own treatment; review at six sessions.

Three rules for this kind of writing. Separate observation from inference, in different sentences. State at least one thing that would prove you wrong. And never write a sentence you would not be willing to read aloud to the family in a feedback session because in many settings, that is exactly what will happen.

4.13 Culture, faith and the boundary question

The concepts in this week are the ones most likely to do cultural damage, because they describe closeness, hierarchy and obligation precisely the domains in which cultures differ most. Four corrections.

Closeness is not enmeshment. In households where three generations share care of children, sleep in the same room, pool income and take decisions collectively, a high level of involvement is functional. The diagnostic question is not "how close are they?" but "can each member think, disagree and move?" A young adult who consults her mother about a marriage proposal is not enmeshed; a young adult who cannot form an opinion without her mother's approval, and who experiences disagreement as betrayal, may be.

Hierarchy is not obedience. A family in which children do not question adults is not automatically dysfunctional. The functional question is whether the hierarchy carries responsibility as well as authority: do the adults protect, provide and decide, or do they merely command and then require the children to manage the consequences?

Filial duty is a resource. Expectations that adult children care for parents and siblings can be a source of meaning and support, and also of exhaustion. Both are true, and the therapist who treats obligation as pathology will lose the family. Ask instead: what does this obligation give you, and what does it cost, and who decided?

Respect for elders can mask abuse. The most difficult clinical situation in this domain is a household in which hierarchy is used to silence a victim: "you cannot speak against your uncle." The professional obligation is clear child protection and adult safeguarding

override deference but the skill is to act without humiliating the family, and to keep the door open for the members who need you. MFT 704 and MFT 724 give the legal and ethical framework; this week's contribution is the conceptual warning.

> **CAUTION**
> "In our culture, families are close" is sometimes a description and sometimes a prohibition on examining anything. The therapist's response is neither agreement nor challenge, but curiosity with boundaries: "Tell me what closeness means in your family, and what happens when someone needs distance."

4.13b A five-minute version for the corridor

Colleagues in hospitals and schools will ask you what you think, in a corridor, with thirty seconds to answer. A usable five-sentence formulation, in the order that survives interruption:

1. **Who is in the system** "Four at home, plus a grandmother who makes the decisions by telephone."
2. **What is happening, behaviourally** "The 16-year-old has been out of school six weeks; her mother answers for her; the father is present but outside the executive."
3. **What maintains it** "Her presence at home reduces her mother's isolation and defers a marital conversation that nobody wants to have."
4. **What I am unsure about** "Whether the father's drinking is a cause or a consequence; I have not assessed it yet."
5. **What I am doing next** "Both parents in the room next week, a joint task on the school letter, and a separate alcohol assessment for the father."

Note the fourth sentence. Including what you do not know is not weakness in a corridor; it is how you prevent a colleague from acting on your guess as though it were an assessment. The habit also protects you: the clinician who always sounds certain is the one who is quoted in a serious case review.

4.13a When the concept is the problem: the misuse audit

A concept becomes dangerous at the moment it stops generating questions and starts closing them. Five signs, each with the question that reopens the inquiry.

Sign 1 the label arrives before the evidence. You find yourself saying "this is an enmeshed family" in the first ten minutes. Reopening question: what exactly did I see, and what else could explain it?

Sign 2 the family's own account is treated as resistance. A mother says "in our family we are close" and you hear denial. Reopening question: what would it mean if she were simply right?

Sign 3 the concept explains everything. Every behaviour in the session becomes evidence of the same structure. Reopening question: name one observation from today that does not fit my formulation.

Sign 4 the concept justifies a plan the family has not agreed to. You find yourself designing an intervention to change a boundary the family has not identified as a problem. Reopening question: whose goal is this, and did they ask for it?

Sign 5 the concept travels into the report as fact. "The family is enmeshed" appears in a letter to a school or a court. Reopening question: can I rewrite this sentence as an observation with a date and a count?

> **AUDIT**
> Take your most recent formulation, written or spoken. Run the five signs against it. Then rewrite it so that every sentence could be defended in a case conference by a colleague who disagrees with you.

4.14 The concept table you will actually use

Twelve concepts, each with what to observe, what it usually costs, and what you would do. Learn this table and you can formulate almost any family in the language the profession shares. Copy it into the front of your clinical notebook.

Concept	What you observe	Usual cost	What you do
Diffuse boundary	Members answer for each other; no privacy; moods shared as weather	Chronic anxiety; difficulty leaving; confidences that a child cannot carry	Mark the behaviour in the room; build one small boundary with the family's consent
Rigid boundary	Topics that cannot be named; formality; information kept in compartments	Isolation; late presentation; secrets	Name the cost of the rule, not the rule; use a third-person question to open it
Weak parental hierarchy	Child decides or vetoes; parents negotiate with the child	Anxious, parentified or defiant child; inconsistent discipline	Give the parents a joint task in the session and coach them through it
Cross-generational coalition	Parent and child allied against the other parent; child knows the marriage	Child loses a parent as a parent; other parent discredited	Re-seat; give the couple a task; support the outsider without blaming the insider
Parentification	Child cooks, translates, manages a parent's feelings	Premature competence; suppressed need; later collapse or resentment	Name the competence and the cost; redistribute one task to an adult
Triangle	Third party discussed whenever two are together; symptom rises with tension	The third person carries what the pair cannot	Ask about the third person's view; bring them in; work the pair's tension directly
Detouring	Couple's conflict reliably becomes a conversation about a child	The child's symptom is the couple's containment	Name the function, cautiously, when the couple can hear it
Rule	Repeated pattern described as custom ("that's just how we are")	Rigidity; invisible obligations	Ask about a specific ordinary occasion; make the rule sayable

Concept	What you observe	Usual cost	What you do
Metarule	"We don't discuss that"; "we don't argue in this house"	The family cannot revise its own rules	Ask what would happen if it were discussed; work with the feared consequence
Role	Stable position (peacemaker, sick one, responsible one)	A person imprisoned in a function	Name the position, not the person; ask who else could occupy it
Myth	Shared belief resistant to evidence ("we never needed anyone")	Reality that cannot be spoken; delayed help-seeking	Work through exceptions and costs, never by confrontation
Secret	Information held by some and withheld from others	Distorted explanations; alliances; a member who "lies"	Ask who is protected, and from what; manage the disclosure ethically

How to use the table in supervision. Present the observation column first, with your evidence, and only then the concept. "The mother answered three questions addressed to the daughter, and the daughter looked at her mother before every answer, so I am working with a diffuse boundary but I may be wrong, because the daughter also corrected her mother twice." A presentation in that order cannot be dismissed as labelling.

4.15 Two families, same concept, opposite interventions

The most important skill in this week is knowing that a concept does not dictate an intervention. Two cases illustrate the point.

Family A the close household. A 20-year-old university student is brought by her parents because she is "withdrawn and rude". She has stopped attending lectures. The family is warm, loud, interdependent. The mother and daughter share a bed when the daughter is home; the father describes his wife and daughter as "best friends". The daughter says, in a private moment, that she has never made a decision without her mother.

Formulation: a diffuse boundary between mother and daughter, which was functional in adolescence and is now costly at the threshold of adulthood.

Intervention: not separation, and emphatically not a lecture on independence. First, name the closeness as a strength and get the family's agreement that it is. Second, create one small experiment: the daughter takes one decision a week without consulting her mother, and reports it afterwards. Third, work with the mother's own anxiety about the daughter's adulthood what does the mother lose if the daughter becomes independent? Fourth, involve the father as a partner to the mother, so that the mother is not left with only the daughter's company.

Family B the distant household. A 20-year-old university student is brought by her parents because she is "withdrawn and rude". She has stopped attending lectures. The family is formal; conversation is conducted in turn; nobody has raised their voice in the house for years. The father works abroad and telephones on Sundays. The daughter says, in a private moment, that she has never told her parents anything that mattered.

Formulation: a rigid boundary around emotional information, with an implicit rule that distress is not discussed. The withdrawal is not individuation; it is the family pattern applied to one member.

Intervention: the opposite of Family A. First, name the absence without accusing anyone: "In this family, people manage things privately is that right?" Second, use a structured, low-risk disclosure task in the session (each person says one thing about the past month that the others did not know). Third, work with the father's distance explicitly, including the question of what he knows about his daughter's life. Fourth, expect the family to find the session uncomfortable and to report that it was "fine" and use that report as the next topic.

The point. Diffuse and rigid boundaries need opposite moves. This is why a diagnosis-like use of concepts produces bad therapy: the same label, applied to two families, licenses interventions that would harm one and help the other. The concept tells you where to look; the family tells you which direction to move.

> **PRACTICAL**
> Read both cases again. Write the first three sentences you would say in each session. Then swap with a peer and check: could your sentences for Family A be said to Family B? If so, you are working from the concept rather than from the family.

4.16 Evidence note: which of these concepts have research support

Students should know how much weight each concept can bear outside the consulting room.

Boundaries and hierarchy. Structural family therapy has a reasonable evidence base for adolescent conduct and substance problems, and observational research on family organisation for example the work on authoritative versus permissive versus authoritarian parenting broadly supports the claim that clear hierarchy with warmth predicts better child outcomes. The specific vocabulary of "enmeshment" has weaker support: measures of family cohesion show curvilinear relationships with wellbeing, which is to say that both very low and very high cohesion can be problematic, and that the optimum depends on culture and stage.

Triangles. Bowen's triangles are theoretically influential and empirically under-tested as a mechanism. There is support for the broader claim that interparental conflict affects children's adjustment, and that children's involvement in parental conflict especially that which makes them feel caught predicts worse outcomes. This is not proof of triangles; it is consistent with them.

Parentification. There is good evidence that destructive parentification (role reversal without recognition or support) predicts internalising problems, and equally good evidence that adaptive or "filial responsibility" in supportive contexts predicts competence. The distinction is now standard in the literature and should be standard in your practice.

Rules, roles, myths. Mostly clinical concepts with qualitative support. They are useful and they are not measurable; say so when you write reports. Their value is descriptive and intervention-generating, not predictive.

Communication concepts. Demand withdraw patterns, negative reciprocity and the balance of positive to negative affect have solid empirical support in couple research. The stronger claims of the early communication school that specific communication deficits cause distress have been revised: communication is as much a product of satisfaction as a cause of it, and the direction of effect is contested.

What this means for practice. Use the concepts; do not over-claim for them. A formulation can be clinically excellent and evidentially modest at the same time, and the professional habit is to be able to say which is which.

4.17 Key terms

Term	Definition
Boundary	The rules governing participation, closeness and privacy between subsystems
Subsystem	A unit within the family organised by generation, function or gender (couple, parents, siblings)
Hierarchy	The arrangement of authority and responsibility, especially across generations
Coalition	An alliance of two against a third that crosses generational lines
Triangle	A three-person emotional configuration recruited to manage tension between two
Detouring	Displacement of couple conflict onto a third party, usually a child
Parentification	A child taking on adult caregiving responsibilities for a parent or siblings
Enmeshment	Diffuse boundaries with excessive sharing and low autonomy (use with care)
Disengagement	Rigid boundaries with little emotional contact
Rule / metarule	A recurring pattern of what is permitted; a rule about rules
Family myth	A shared belief about the family that organises behaviour and resists disconfirmation
Metacommunication	Communication about the relationship or about communication itself
Symmetrical / complementary	Equal versus different-and-mutually-fitting exchanges
Genogram	A three-generation diagram of structure, relationships and events
Homeostasis	The system's tendency to maintain its existing organisation
Function of a symptom	The work a symptom does in maintaining the system's organisation

4.18 Self-test

1. Name the three boundary types and give one observational marker of each.
2. What distinguishes a coalition from a triangle?
3. What is the difference between detouring-attacking and detouring-supportive?

4. Why is "enmeshment" dangerous as a label in a collectivist household?
5. What is a metarule, and how do you surface one?
6. Which question helps you decide whether a secret is protective?
7. Give one concept from this week with strong empirical support and one with weak support.
8. Why do two families with the same boundary problem need opposite interventions?

Answers. 1. Clear (each member has voice and privacy; adults decide adult matters), diffuse (members answer for each other; moods shared as weather), rigid (topics that cannot be named; formality). 2. A coalition is a structural alliance across generations against a third; a triangle is an emotional process in which a third is recruited to manage tension between two. 3. In detouring-attacking the child is defined as the problem and the parents unite against them; in detouring-supportive the child is defined as fragile and the parents unite to protect them. 4. Because high involvement can be functional and normative; the diagnostic question is whether members can think, disagree and move, not how close they are. 5. A rule about what may be discussed; surface it by asking what would happen if the topic were raised. 6. Ask who is protected by the secret and who is protected from it. 7. Hierarchy/parenting style or demand withdraw has reasonable support; "enmeshment" as a diagnosis or Bowen's triangles as a mechanism have weak support. 8. Because the same label can describe under-involvement or over-involvement, and the direction of the required change is opposite.

4.19 Reflective task

Draw your own family of origin as a structural map: subsystems, boundaries, hierarchy, one triangle, one rule that was never stated. Then answer three questions in 300 words. Which of the concepts was easiest to apply to your own family, and why might that be? Which was hardest, and what does the difficulty protect? And what would your family say if they saw the drawing?

Bring the drawing to the lab, but not the answers to the third question: that one stays private, and you will return to it in Week 14.

4.21 Appendix: the case bank for this week

The lab draws on four cases. Read all four; you will be assigned one.

Case 1 the returning father. A father returns to the household after three years working in another city. The 13-year-old son refuses to obey him and the mother mediates every exchange. Concept focus: hierarchy, coalition, re-entry.

Case 2 the two-household child. A 10-year-old moves between two households with different rules, and presents with "behaviour problems" that occur only after each transition. Concept focus: boundaries between systems, rules across contexts, the child as the carrier of a conflict between two subsystems.

Case 3 the bereaved couple. A couple in their fifties lost their eldest son eighteen months ago. They describe no conflict and no intimacy. Their younger daughter has

become the household's organiser. Concept focus: rigid boundary around grief, parentification, the function of silence.

Case 4 the church leader's family. A pastor's household in which the father is a public figure and the family's difficulties are treated as a threat to his ministry. The 17-year-old daughter has disclosed self-harm to a teacher. Concept focus: myth, secret, hierarchy, and the point at which risk overrides the family's preferred account.

What to produce for your assigned case. A genogram, a structural map, a one-paragraph formulation, two competing hypotheses with the question that would test each, and the first intervention with its rationale. Two pages maximum. The two-page limit is deliberate: a formulation you cannot compress is one you have not finished thinking about.

How the four cases are marked together. Cases 1 and 2 test hierarchy and cross-system boundaries; cases 3 and 4 test the concepts that carry the most risk of harm parentification and secrecy respectively. Present your case to a group working on a different one, and spend five minutes on the question: which of your concepts would have been dangerous in the other group's family, and why? That question, asked honestly, is the best protection against the misuse discussed in section 4.13a.

4.20 Readings for this week

- **Minuchin (1974)**, Families and Family Therapy, chapters 2 4. Boundaries, subsystems, mapping.
- **Bowen (1978)**, Family Therapy in Clinical Practice, chapter 8 on triangles. Read for the mechanism, not the dogma.
- **Nichols & Schwartz**, chapters 5 6. The structural model, summarised fairly.
- **Byng-Hall (1995)**, "Creating a secure family base", Family Process. On caregiving roles and their costs a bridge between structure and attachment.
- **Kerig (2005)**, on parentification. Read one empirical paper and notice the distinction between destructive and adaptive forms.
- **Palazzoli et al. (1978)**, Paradox and Counterparadox, on the referring person and the function of the symptom. Dated and illuminating.

Next: Week 5 The profession: identity, scope and settings

Week 5 The profession: identity, scope and settings

5.1 The lecture in ninety minutes

Students spend the first month learning to think relationally. This week asks a different question: who are you when you do it? Not in a philosophical sense, but in the practical one that governs your career what you are licensed to do, where you may do it, what you must refuse, how you are paid, who you answer to, and how you explain yourself to a judge, a school principal, a church council or a hospital consultant.

The week has an uncomfortable edge, and it should. Family therapy is a young profession in most jurisdictions, practised by people with several different trainings, sometimes without a clear legal footing. Students who imagine that professional identity is conferred automatically by a certificate will be unprepared for the questions that arrive in practice: Can I see this child without the father's consent? Does my report carry weight in court? May I call myself a psychologist? What do I do when the imam who referred this family expects a report on their spiritual state?

Minutes	Segment	What the lecturer does	What students do
0 12	What makes an occupation a profession	Six criteria, applied to three occupations	Score MFT against the six
12 30	Scope of practice	The three circles: competence, authorisation, mandate	Place ten tasks inside or outside
30 45	Training and registration	Routes into the work; what each title may and may not claim	Map their own route and its limits
45 60	Settings	Six settings, each with its own contract	Identify the invisible contract in a case
60 75	Teams, referrals and boundaries	A live referral letter, criticised	Draft a referral in eight lines
75 90	Money, records and the long career	Fees, sliding scales, notes, supervision, burnout	Write their own three-year CPD plan

5.2 What makes an occupation a profession

The sociology of the professions gives six criteria. They are not a definition to be recited; they are a checklist for understanding the negotiations that shape your working life.

1. A specialised body of knowledge. Not merely skills, but a system of theory that practitioners must learn and that outsiders cannot easily acquire. Family therapy has this: systems theory, cybernetics, structural and attachment-based models, and a distinctive literature. The weakness is that the knowledge is dispersed across several disciplines, which makes the claim to exclusivity harder than it is for medicine.

2. Prolonged training and socialisation. A profession reproduces itself through education that changes who the student is, not only what they know. The reflective tasks in this library are part of that socialisation, not an indulgence.

3. A code of ethics with enforcement. Not a statement of values on a website, but a mechanism: a code, a complaints route, and sanctions including removal. Where the enforcement mechanism is weak, the public is protected mainly by individual conscience, which is not a system.

4. A registration or licensing mechanism. Some legal recognition of who may use the title and practise the activity. This is where the picture varies enormously across jurisdictions, and where students must know the facts for their own country rather than assume.

5. Autonomy and self-regulation. The profession, not the state or the market, sets its standards. Autonomy is always partial and always contested by insurers, by employers, by government and it has costs: professions that regulate themselves are slow to discipline their members.

6. A service orientation with a fiduciary duty. The client's interest comes before the practitioner's. This is the criterion most easily damaged by the conditions of practice: a therapist paid per session by an agency that requires a diagnosis has a conflict that must be managed openly, not denied.

Where family therapy sits. Strong on 1, 2 and 3; variable on 4 and 5 depending on jurisdiction; and permanently at risk on 6 wherever funding structures reward volume over outcome. A student who understands this will not be surprised by the political arguments that surround the field, and will know which of them they can influence and which they cannot.

> **AUDIT**
> Score marriage and family therapy, in your own jurisdiction, against the six criteria, 0 2 each. Where you score 0 or 1, write the name of the body if any that could change it. Most students discover that the weakest criterion is registration, and that the second weakest is enforcement.

5.3 Scope of practice: three circles, not one

The commonest professional error is to equate scope of practice with competence. It is narrower and more complicated. Think of three circles.

Circle 1 Competence. What you can actually do well, given your training, your supervised experience and your current skills. This is a factual question, and the honest answer changes over a career: it grows with deliberate practice and shrinks with neglect.

Circle 2 Authorisation. What you are permitted to do by law, by your registering body, and by your employer. A competent family therapist may be legally unable to write the report a court requires, or to see a child without a guardian's consent, or to assess for a diagnosis that only a clinical psychologist may make in that jurisdiction.

Circle 3 Mandate. What this particular client, family or referrer has asked you to do, and what the setting expects. A school that refers a child wants an opinion about schooling; a court wants an opinion about custody; a family wants relief. These are not the same mandate, and the therapist who does not clarify it will eventually be experienced as either useless or intrusive.

The safe zone is the intersection. Competent, authorised and mandated. When one circle is missing, three legitimate options exist: refer, consult and work within limits, or obtain additional training and authorisation before proceeding. The illegitimate option which is common, well-meant and career-ending is to proceed because the family needs help and nobody else is available.

Situation	Circle missing	Correct response
Asked to assess a 6-year-old for ADHD and recommend medication	Authorisation (prescribing) and probably competence	Assess the family context, write to the paediatrician, do not recommend medication
Asked by a court to say which parent should have custody	Authorisation (in many jurisdictions) and competence (forensic)	Offer a clinical opinion on the child's needs; recommend a forensic assessment
Asked to see a couple where there is current violence	Competence and safety conditions	Screen, safety-plan, consider separate sessions, refer to a specialist service
Asked by a church to "deliver" a family from a spiritual problem	Mandate mismatch	Clarify roles; offer clinical work alongside, not instead of, pastoral care
Asked to work with a family in a language you speak poorly	Competence	Use a trained interpreter; do not use the family's child

5.4 Training, titles and what they may claim

Titles matter because they carry legal and public meaning. The details vary by country and change over time; what follows is orientation, and the rule is always: **verify the current requirements with the relevant council or association before you use a title or accept a referral.**

The routes into the work, in outline.

- **Marriage and family therapy as a distinct qualification** a master's or doctoral programme in MFT, usually with supervised clinical hours, a placement, and a research component. This is the route this library supports.
- **Psychology** clinical or counselling psychology, with its own professional registration, assessment privileges and, in many jurisdictions, a protected title.
- **Counselling** a large and varied field, ranging from rigorous postgraduate training with supervised practice to short courses. The variance is the problem: a title that can mean anything protects nobody.
- **Psychiatry** a medical route, with prescribing rights and, usually, authority over admission and diagnosis in institutional settings.
- **Social work** with statutory powers in child protection in many jurisdictions, and a distinctive ecological training.
- **Pastoral and faith-based counselling** substantial in practice, variable in training, and often the first service a family encounters.

Three things to be able to say about your own training. What you are qualified to do; what you are not; and what you are working towards. The third is what distinguishes a professional from a technician. Write the three sentences now, and revise them every year.

The title trap. Students sometimes use a title loosely "psychotherapist", "counselling psychologist", "family therapist" in a jurisdiction where the title is protected or where

an employer interprets it narrowly. The consequences range from embarrassment to prosecution, and they always damage the client's confidence. Use the title your registration permits, and describe the work rather than the label: "I work with couples and families using a systemic approach" is accurate, informative and safe.

> **CAUTION**
> Never describe yourself as supervising, diagnosing or assessing beyond your authorisation, even informally and even when asked by a senior colleague. The sentence "as the family therapist, I would say this child has " has ended careers, and more importantly has harmed families who acted on an opinion that had no standing.

5.5 Six settings and their invisible contracts

Each setting imposes a contract that nobody writes down. The therapist who does not read it will be in conflict with the institution within a month.

Outpatient clinic. The contract: the family self-refers or is referred; attendance is voluntary; the work is time-limited by funding. What this affords: the best conditions for systemic work a room, a series of sessions, a family that chose to come. What it demands: rapid engagement, because dropout is highest in the first three sessions. The characteristic failure: treating a voluntary contract as though it were compulsory, and losing the family by session two.

Hospital inpatient and liaison. The contract: the patient is admitted; the family is a visitor; the medical team owns the plan. What this affords: access at a moment of crisis, when a family is most open to change. What it demands: brevity, clarity about what you are and are not deciding, and tolerance of hierarchy. The characteristic failure: conducting a leisurely family assessment while a consultant waits for a one-paragraph opinion.

School. The contract: the school has a duty to the child's education and to other pupils; the parents have rights; the child has a voice that is easily lost. What this affords: early intervention, and access to the system in which the symptom is most visible. What it demands: an explicit agreement about what will be shared with whom, in writing, before the first session. The characteristic failure: becoming an instrument of institutional discipline, so that the child experiences therapy as another arm of the school.

NGO and humanitarian settings. The contract: a donor funds a defined service with defined indicators; the population is often displaced, bereaved and without legal documentation. What this affords: reach and flexibility home visits, groups, community workers. What it demands: attention to the difference between what the donor counts and what the family needs, and a firm grip on boundaries in a context where the therapist may be the only professional the family sees. The characteristic failure: imposing a manualised model designed elsewhere, and recording outcomes that satisfy the indicator rather than describe the family.

Faith settings. The contract: the community's authority is spiritual; the therapist may be a guest, an employee or a member. What this affords: trust, access, and a language of

meaning that many families prefer to the language of pathology. What it demands: explicit role clarity who is responsible for what and willingness to name the limit when a spiritual explanation is being used to avoid medical or protective action. The characteristic failure: two authorities giving contradictory instructions to a family, with the family caught between them.

Court, statutory and forensic. The contract: the court is the client; the family is the subject; your opinion may be tested under adversarial conditions. What this affords: influence at the point where a family's life is decided. What it demands: forensic competence, meticulous records, and a clear statement of the limits of your assessment. The characteristic failure: the therapist who has been treating the family writing a custody opinion without the training, the mandate or the methodology.

> **PRACTICAL**
> Choose one setting you expect to work in. Write its invisible contract in five lines: who pays, who refers, who owns the plan, what is recorded, who can see the record. Then write the sentence you would say to a family in the first session to make that contract explicit.

5.6 A day in three settings

The abstractions become concrete in the ordinary texture of a working day. Three Thursdays, three settings.

Thursday in an outpatient clinic. Eight appointments, three of them new. The first family cancels; you use the hour to write a report that was due yesterday. The second is a couple in their forties, together eleven years, presenting with "we argue about everything"; by minute twenty you have the cycle, by minute forty they have agreed on one experiment, and you have agreed to see them fortnightly because they can only afford that. The third is a 15-year-old who comes alone because her mother works; you spend ten minutes on consent, and establish that the mother has agreed to the referral in writing. Between appointments you take a call from a school counsellor who wants to know whether a pupil's self-harm is "serious enough" to refer; you say you cannot discuss a pupil you have never met, and offer a consultation with the counsellor about the process. You finish twenty minutes late, which is normal and is a warning sign.

Thursday in a hospital. You are asked to see a family on the medical ward: a 62-year-old man after a stroke, his wife, two adult daughters, one of whom has travelled from abroad. The medical team wants to know whether the family "will manage at home". You have one hour and a ward round at three. You do four things: you find out what each person has understood of the diagnosis (they have four different understandings); you ask who makes decisions and who will do the caring (everyone assumes the daughter who lives nearby, who has not been asked); you name the disagreement between the sisters about nursing-home care, which nobody has said aloud; and you write six lines in the notes with a plan and a named person responsible for each step. The consultant reads it and says "useful", which in a hospital is high praise.

Thursday in a faith-based counselling centre. Three couples, all referred by their pastor. The first is straightforward couple work. The second involves a husband who has been violent twice; the pastoral expectation is reconciliation, your assessment is that the wife is at risk, and you spend the session on safety planning and on being explicit, in the pastor's presence, about what you will and will not do. The third is a family convinced that the daughter's seizures are spiritual; you establish that she has had a neurological workup, that the medication is being given irregularly because of the family's beliefs, and you negotiate carefully, with the pastor as an ally rather than an obstacle a plan in which prayer continues and the medication is given.

What the three days have in common. Every session involved a negotiation about role, mandate or confidentiality before any clinical work happened. The clinical content was, in each case, easier than the contract. This is what experienced practitioners mean when they say that most of the job is boundaries.

5.7 Working in a multidisciplinary team

Family therapists rarely work alone. The team is where the profession's identity is tested, because the team carries other professions' assumptions about what a family therapist does.

Four practical disciplines.

1. **Write your role into the record.** "Family meeting held; systemic assessment of the family's understanding of the diagnosis and capacity to care at home; recommendations below" tells the team what you did and did not do. "Family seen" tells them nothing and invites them to assume you did whatever they imagine.
2. **Separate information from opinion.** In team meetings, use two sentences: "What I observed was " and "What I think it means is ". Colleagues will quote the first as fact and the second as your view, which is correct, and your practice should make the difference audible.
3. **Protect the family's confidences within the team.** The team needs the information necessary for care, not everything you know. A wife's disclosure of an affair is not team business unless it bears on safety; a child's disclosure of self-harm is team business immediately.
4. **Refuse the role of the team's conscience.** You will be invited to be the person who raises the emotional dimension that nobody else wants to discuss. Accept the task, not the identity: raise the issue, name the evidence, propose an action, and let the team decide.

Three conflicts that recur, and how to handle them.

The diagnosis conflict. A psychiatrist diagnoses the child and the team organises treatment around the child; you think the family is maintaining the problem. You are probably partly right and partly wrong, and the family needs both. Handle it by agreeing a shared formulation in writing, with each profession's contribution named, rather than by arguing about whose model is true.

The confidentiality conflict. The team wants information that the family gave you in confidence. Handle it by telling the family, at the start, exactly what you will share with the team and why and by returning to them before you share anything outside that agreement.

The authority conflict. A senior colleague instructs you to do something outside your scope. Handle it by naming the scope, offering the alternative, and putting the exchange in writing, courteously. If the instruction is repeated, it becomes a supervision and, if necessary, a registration matter.

> **CASE**
> A paediatric team asks you to "sort out the mother" of a child with a chronic condition. The referral assumes she is the problem: she is "anxious, over-involved, not following the plan". You meet her. She is a woman managing a child's illness, two other children, a job and a husband who is away; she is following the plan as she understands it, and the plan was explained to her once, in English, at a bedside, while her child was crying. The team's formulation was not malicious; it was the natural product of a system that had never asked her what she had understood. Your intervention is not to treat her anxiety. It is to write a note that records what she understood, what she was asked to do, and what support she would need and to ask the team to repeat the explanation with her, in her language, with a written summary.

5.8 Referral: the skill nobody teaches

A referral is a clinical act. Done well, it transfers care without losing the family; done badly, it loses them, or it transmits a formulation that follows the family for years.

The eight lines of a good referral. Every referral letter or call should contain, in this order:

1. **Who you are and what you are asking for.** "I am the family therapist seeing the Adeyemi family; I am asking for an alcohol assessment, not for treatment to begin without them."
2. **The presenting problem, in the family's language and in yours.** Both, labelled.
3. **What you have tried and what happened.**
4. **What you have assessed, and how.** Name the instruments and the sessions.
5. **Risk, stated explicitly.** Not "no concerns" say what you asked about and what was reported.
6. **What the family has been told about the referral, and whether they agree.** Never refer behind a family's back except where safeguarding requires it, and then say so.
7. **What you are asking the recipient to do, and by when.**
8. **Your contact details and your availability for a conversation.**

What families experience. A referral is often experienced as rejection: "she has given up on us". Two sentences prevent this. First, name the referral as an addition rather than

an ending: "I am not stopping work with you; I am bringing in someone who knows more about this than I do." Second, be specific about what continues: "we will still meet on the 14th, and I want to hear how the appointment went."

Referral failures worth recognising.

- **The hot potato.** A family referred to four services in a year, each of which assessed them and passed them on. The pattern is usually a sign that the presenting problem is relational and each service is looking for an individual diagnosis. The systemic response is to convene a meeting of the involved professionals with the family's consent so that the network is treated as a system too.
- **The referral that transmits a verdict.** "Mother is enmeshed and resistant" in a referral letter becomes the next clinician's premise. Write observations, not verdicts.
- **The referral that abandons.** A family referred to a service with a two-year waiting list, and no interim plan. If the wait is long, the referral needs a holding plan, and the family needs to be told what to do if things get worse.
- **The referral to nowhere.** A service that does not exist, or does not take that kind of case. Check before you promise.

5.9 Money, contracts and the ordinary ethics of practice

Therapy is a service delivered in an economy. Students are often taught to be embarrassed by this, which produces two failure modes: therapists who cannot discuss money and therefore resent their clients, and therapists who optimise for income and call it efficiency.

Fees. Three principles. First, the fee is agreed before the work begins, in writing, including what happens to missed sessions. Second, the fee should be one you can charge without resentment: a therapist who feels underpaid becomes subtly punitive. Third, sliding scales and pro bono work are legitimate and should be structured rather than improvised a fixed number of reduced-fee places, reviewed at intervals, protects both the client and the therapist.

Missed sessions. The policy should be simple, written, and applied consistently: for example, 48 hours' notice, otherwise the session is charged; exceptions for emergencies, at the therapist's discretion. The point is not the money; it is that an unclear policy produces resentment, and resentment produces bad therapy.

Records. Keep notes that are accurate, dated, and written as though the family and a court will read them because both may. Record what was observed, what was said, what was decided, what was risk-assessed, and what the plan is. Do not record speculation as fact, do not record your irritation, and do not record information that is irrelevant to care. Retention periods and access rights are jurisdiction-specific: know yours, and tell families at the start.

Contracts with third parties. When an employer, a school, a court or a donor pays, the client is not the only party with a claim. The professional answer is not to pretend otherwise but to write it down: who the client is, what will be shared, what will not, and

what happens if the interests diverge. Families can understand this if it is said plainly in the first session; they cannot forgive discovering it later.

> **PRACTICAL**
> Write your own practice policy on one page: fees, missed sessions, confidentiality and its limits, records and retention, contact between sessions, and what you do in an emergency. Show it to a peer and ask them to find the three sentences a distressed family would misread. Rewrite those three.

5.10 The law, at the level a therapist needs

Legal detail belongs to MFT 704 and MFT 724. What belongs here is the shape of the therapist's legal exposure, because it determines how you practise in a setting.

Consent. Sessions with minors require the consent of a person with parental responsibility, except in specific statutory situations. Separated parents may each hold rights; a therapist who assumes the custodial parent speaks for both can find themselves in a dispute they did not anticipate. Establish, in writing, who has consented and who has been informed.

Confidentiality and its limits. Every family should be told, in the first session, in plain language, the circumstances in which you would share information without their consent: risk of serious harm to a child or vulnerable adult, risk of serious harm to self or others, and where a court orders disclosure. Then and this is the part students forget you must be able to say what you would do, not merely that there are limits.

Records and disclosure. Notes can be requested by courts, insurers, employers and the family themselves. Write accordingly. Where a request arrives, the correct response is rarely immediate compliance or immediate refusal: it is to seek advice, to inform the family where lawful, and to disclose only what is required.

Reports. A report written for a court or a school is a professional document with consequences. Three rules: state your qualifications and the limits of your assessment; separate observation from opinion; and never express an opinion outside your competence, however strongly you are pressed.

Duty to report. Many jurisdictions impose mandatory reporting of suspected child abuse. Know whether yours does, know the threshold, and know that a therapist's own judgement about whether a report will "help the family" is not a lawful basis for withholding one.

5.11 The long career: supervision, CPD and burnout

The profession's most common casualty is not malpractice but attrition. Therapists leave because they are exhausted, isolated, under-supported and under-paid, and the families who need experienced clinicians lose them.

Supervision. Not optional, not a sign of deficiency, and not the same as management. A supervisor is responsible for the quality and safety of your work; the relationship should

be contracted explicitly frequency, cost, what is recorded, what happens in a disagreement, and how the supervisor would respond to a concern about your practice. Group supervision adds perspective and costs less, but individual supervision is necessary at points of difficulty. MFT 720 covers the practice in depth.

Continuing professional development. A three-part plan is more useful than a course list: one depth activity (a long training in a model you use), one breadth activity (something outside your model, to keep you honest), and one evidence activity (reading primary research in your area). Review annually. The trap is accumulating certificates in fashionable approaches while never deepening a single competence.

Burnout, and how it actually presents. Not as cynicism alone. Early signs: sessions feel longer; you stop writing notes properly; you notice you are avoiding a particular family; the families begin to blur; you are irritable at home; you feel relief when someone cancels. Later signs: dread before sessions, sleep disturbance, loss of empathy, intrusive thoughts about a case, and a private conviction that you are fraud.

What actually helps. Caseload limits you can defend; a peer group that meets reliably; supervision that is about the work rather than about your personality; attention to the practical (sleep, money, commuting); personal therapy when the material is touching your own history; and the professional courage to say "I cannot take this case" before you take it.

> **CAUTION**
> Vicarious trauma is an occupational hazard, not a character weakness, and the therapists most at risk are often the most conscientious. If you notice yourself avoiding a case, that is data. Take it to supervision before it becomes a decision you make alone.

5.12 Annotated transcript: the contract conversation

The first five minutes of a first session, in a faith-based setting, where the pastor has referred the family and is present at the start.

> **T1 Therapist:** Before I ask about the difficulty, I want to be clear about who I am and what happens to what you tell me. May I take five minutes on that?
> **T2 Father:** (to the pastor) She is the one you said would help.
> **T3 Therapist:** I am. And I want to be honest about my part. I am a family therapist; I am not a pastor, and I will not lead prayers. Pastor, you are here at the start because you made the referral, and my suggestion is that after today you are not in the room, unless the family wants you there for a particular session.
> **T4 Pastor:** That is fine. I only want to know that they are being helped.
> **T5 Therapist:** I will tell you whether they are attending and whether the plan is being followed. I will not tell you what they say about each other, and I will not give you a report. Is that acceptable to everyone?
> **T6 Mother:** (quietly) Yes.
> **T7 Therapist:** Two more things. If I believe a child is at risk of harm, I will have to act, and I will tell you what I am doing and why. And my notes are my records; you may ask to see what I have written about you.

Annotations. T1 asks permission for the contract conversation, which is both courteous and structural: it signals that the therapist has a procedure and is not purely reactive. T3 does three things at once states the professional identity, declines the pastoral role explicitly (which respects the pastor rather than competing with him), and proposes a boundary for the pastor's presence. Note that the boundary is proposed, not imposed: "my suggestion is".

T5 is the most important sentence in the exchange. It distinguishes attendance and adherence (legitimately shared with a referrer) from content (not shared without consent). Most boundary failures with referrers are failures to make exactly this distinction.

T6 the mother's quiet "yes" should be noticed. The therapist does not pursue it in front of the pastor, but notes it and returns to it in the first individual moment available. The quietest voice in a contract conversation is often the one who will later disclose the most serious material.

T7 states the limits of confidentiality and records in two sentences, in plain language, before the family has said anything they might regret. Students often leave this to the end of the session, when the family is committed and the disclosure has already happened; that ordering is both less ethical and less effective.

5.13 Laboratory: the scope-of-practice tribunal

A ninety-minute exercise in which students practise refusing, referring and negotiating the three skills nobody enjoys practising and every professional needs.

Setup. Groups of four. Three students receive a scenario card; one is the "registrar's assessor". Each scenario contains a plausible, urgent request that exceeds the therapist's scope, plus pressure to comply (a senior colleague, a distressed family, a deadline).

Scenario examples.

- A school principal asks you to confirm in writing that a pupil is "not a risk" so that he may return to school tomorrow, after an incident you did not witness and a family you have met once.
- A mother asks you to see her 14-year-old without telling the father, from whom she is separated.
- A judge's clerk asks you to send your case notes by Friday, including notes of individual sessions.
- A pastor asks you to tell him whether the wife in a couple you are seeing is "telling the truth" about her husband's violence.
- A hospital consultant asks you to "sign off" a family's capacity to care for a dying relative at home.

The task. The therapist must: identify which circle (competence, authorisation, mandate) is missing; say what they can offer; make one referral or consultation; and put the essential exchange into two written sentences. Six minutes per scenario, then the assessor gives a verdict.

Marking rubric.

Criterion	Weight	Full marks
Identification of the missing circle	25%	Names competence, authorisation or mandate precisely, not vaguely
Offering an alternative	25%	Offers something specific and useful; does not merely refuse
Boundary language	20%	Courteous, brief, no over-explanation, no moralising
Written record	20%	Two sentences that would protect the family and the therapist
Pressure handling	10%	Holds the boundary under at least two escalations without becoming defensive

5.14 Six errors in professional identity

1. **The competence-authorisation confusion.** Doing something you are good at but not permitted to do. Fix: know your three circles and check them before you accept a referral.
2. **The invisible contract.** Working in a setting without ever making its terms explicit to the family. Fix: the five-line contract conversation, in the first session, every time.
3. **The ambitious referral.** Promising a service you have not confirmed exists. Fix: check, then promise.
4. **The title stretch.** Using a protected or ambiguous title because it sounds more credible. Fix: describe the work, not the label.
5. **The solo practitioner.** Working without supervision, peer contact or a route to advice. Fix: build the network before you need it; the moment you need it is too late to build it.

6. **The unpaid martyr.** Accepting a caseload that cannot be sustained, then resenting the families. Fix: a written caseload policy and the discipline to say no.

5.15 Culture, faith and the professional's own position

Three observations that belong to this week rather than to the culture course, because they are about the profession's identity rather than the family's.

The imported profession. In many countries the structures of professional practice registration, title protection, insurance, supervision contracts were modelled on Northern institutions and do not map cleanly onto local realities of practice, funding and authority. A therapist working in a context where most families see a pastor first, where few can afford private fees, and where the state provides little, is practising a profession whose formal infrastructure is thin. The honest response is neither cynicism nor imitation: it is to build what is missing locally peer supervision groups, referral networks, written policies, outcome measurement rather than waiting for a regulator to arrive.

The class position of the therapist. You will usually be more educated and better paid than the families you see, and you will be consulting them in a room they had to travel to. This asymmetry is not dissolved by friendliness. It is managed by transparency: explaining what you are doing and why, offering choices, naming your own limits, and refusing to make decisions that belong to the family.

Faith and the therapist's own convictions. A therapist with strong religious commitments will meet families whose beliefs differ, and a therapist with none will meet families for whom faith is the organising fact of life. Both need the same discipline: the family's beliefs are part of the system you are assessing, not a position you are required to endorse or correct. If a family's belief leads to a child being denied medical care, your obligation is to act under the law and safeguarding framework and to do so in a way that keeps the family in contact with help rather than driving them underground.

> **REFLECT**
> Write 300 words on the kind of professional you intend to be in five years: what you will be competent in, what you will refuse, who you will consult, and how you will know you are still learning. Keep it; you will compare it with reality at the end of the programme.

5.16 Evidence note: does professionalisation improve outcomes?

A question students rarely ask and should: does all this registration, codes, supervision, CPD actually make therapy better for families?

What the evidence suggests. Therapist effects are large: in most psychotherapy datasets, a small proportion of therapists account for a disproportionate share of outcomes, and the variance attributable to the therapist exceeds that attributable to the model. Training and deliberate practice predict skill acquisition; experience alone does not reliably predict better outcomes, and some therapists decline over time. Routine outcome monitoring, with feedback to the therapist, improves outcomes and reduces deterioration, particularly for therapists whose clients are deteriorating. Supervision is

mandated almost everywhere and is much less studied than its ubiquity would suggest; the available evidence links good supervisory alliance to better outcomes, and links supervision that is punitive or absent to worse ones.

What this implies for professional structures. The mechanisms most likely to protect the public are not titles but practices: measurement of outcomes, feedback loops, supervision with a genuine authority to intervene, and a culture in which a therapist can say "this case is not going well" without career damage. Registration without these practices protects the profession's status more than it protects families.

What students should demand of their own training. Regular outcome measurement in your placements; supervision that watches your actual work, not only your account of it; and honest discussion of your failures. If your training does not provide these, the professional habit to build is to provide them for yourself.

5.17 Key terms

Term	Definition
Profession	An occupation with specialised knowledge, prolonged training, a code, enforcement, and a service orientation
Registration / licensure	Legal recognition of who may use a title and practise an activity
Scope of practice	The intersection of competence, authorisation and mandate
Mandate	What the client, referrer or setting has asked you to do
Fiduciary duty	The obligation to put the client's interest before your own
Duty to report	A statutory obligation, in some jurisdictions, to report suspected abuse
Parental responsibility	The legal authority to consent on a child's behalf
Sliding scale	A structured reduced-fee arrangement
Vicarious trauma	Cumulative harm to the therapist from exposure to clients' trauma
Burnout	Exhaustion, detachment and reduced efficacy resulting from chronic occupational stress
Therapist effects	Variance in outcome attributable to the individual therapist rather than the model
Routine outcome monitoring	Session-by-session measurement of outcome and alliance, fed back to the therapist

5.18 Self-test

1. Name the six criteria of a profession and identify the two weakest for family therapy in most jurisdictions.
2. A family asks you to testify that a father is "safe" so that a court will allow unsupervised contact. Which circle is missing, and what do you offer?
3. What is the difference between competence and authorisation? Give an example of each.

4. What should a referral letter contain? Name five of the eight elements.
5. Why is a written missed-session policy a clinical matter and not merely an administrative one?
6. What is the strongest evidence-based argument for routine outcome monitoring?
7. What are the early signs of burnout, and what is the first thing you should do?
8. Why might registration alone not protect the public?

Answers. 1. Specialised knowledge; prolonged training; code with enforcement; registration; autonomy; service orientation. Weakest: registration and enforcement. 2. Authorisation (and forensic competence); offer a clinical opinion on the child's needs and recommend a forensic assessment. 3. Competence is what you can do well; authorisation is what you are permitted to do for example, being skilled at assessment but not authorised to diagnose in that jurisdiction. 4. Any five of: who you are and the request; the presenting problem in both languages; what you tried; what you assessed and how; risk; what the family has been told; what you want and by when; contact details. 5. Because an unclear policy produces resentment, and resentment is enacted in the therapy. 6. Feedback improves outcomes and reduces deterioration, especially for therapists whose clients are worsening. 7. Sessions feel long, notes slip, avoidance of a particular family, families blurring, irritability, relief at cancellations; take it to supervision immediately. 8. Because titles do not change practice; outcome measurement, supervision with real authority, and a culture that tolerates admitting failure do.

5.19 Readings for this week

- **AAMFT Code of Ethics** (current edition). Read it as a document about power, not as a list of rules.
- **Corey, Corey & Callanan**, Issues and Ethics in the Helping Professions, chapters on competence and boundaries. The standard orientation text.
- **Strupp & Anderson (1997)**, on the therapist as a person, and the contemporary literature on therapist effects (for example the reviews by Wampold and by Barkham). Why who you are matters as much as what you do.
- **Lambert et al.**, on routine outcome monitoring and feedback. The strongest practical case for measurement.
- **Local professional association guidance** for your jurisdiction: registration requirements, codes, complaint procedures. Read the actual documents rather than the summaries.
- **Roberts (2005)**, Crisis Intervention Handbook, chapter on settings useful on institutional contracts.
- **Your own jurisdiction's registration and complaints guidance.** Read the complaints procedure, not only the code of ethics: it tells you what the public can actually expect of you, what kind of failure is treated as serious, and how long a determination takes. Students are routinely surprised by both the scope and the pace of the process.

Next: Week 6 The first session I: joining

5.5a Two more settings: private practice and telehealth

Two settings have grown fast enough to need their own note, because each changes the professional contract in ways that catch newly qualified therapists unprepared.

Private practice. The contract: the family pays directly, self-refers, and may leave at any time. What this affords: continuity, flexibility, and freedom from institutional definitions of the problem. What it demands: a business infrastructure a written agreement, a fee policy, a records system, insurance, a referral network, and a plan for what happens when the therapist is ill or on leave. The characteristic failures: isolation (no team, no supervision, nobody to notice deterioration); over-acceptance (taking every referral because income is uncertain, and ending up with a caseload beyond competence); and the boundary erosion that comes from being reachable by WhatsApp at all hours. The professional answers are unglamorous and effective: a fixed number of clients, a supervision contract regardless of whether anyone requires it, an answering policy that states your response times, and a rule that clinical content is discussed in sessions and not in messages.

Telehealth. The contract: the same clinical relationship, delivered through a medium that changes what can be seen and who can be present. What this affords: access for families separated by distance or work, and a window into homes that a clinic never provides you will see who walks through the room, who is listening off-camera, and what the household actually looks like. What it demands: explicit work on privacy (who else is in the house, who can overhear), on safety (you cannot physically intervene; you need a local contact and an address at the start of every session), on consent (recording, and the limits of confidentiality on a platform), and on the technical frame (a plan for disconnection). The characteristic failure: treating a video session as though it were a clinic session with a camera on it, and missing both the advantages and the risks.

A practical telehealth checklist for the first session. Confirm the full address of each participant; ask who else is in the house; agree a signal for "I cannot speak freely"; agree what happens if the connection drops; confirm that the family understands the limits of emergency response at a distance; and record the consent. Ten minutes of setup prevents the two most serious telehealth failures: a disclosure overheard by someone who should not have heard it, and a crisis in which the therapist does not know where the family is.

5.7a The network as a system: case management when several services are involved

Families with complex difficulties are usually known to several services at once: a school, a clinic, a social worker, a church, sometimes a court. This network behaves like a system, with the same properties you learned in Week 4 boundaries, rules, triangles, and a tendency to nominate one member as the problem.

The observable signs that a network has become a system problem. Professionals criticise each other in front of the family; the family tells each professional a different story and each believes theirs; there are four assessments and no plan; the family is described by every service in the same negative terms ("non-engaged", "chaotic",

"resistant"), which is usually a description of the network's failure rather than the family's; and meetings are held about the family rather than with them.

The intervention. A network meeting, convened with the family present, with four agenda items and nothing else: what each service has observed; what each service is doing; what the family wants; and who will do what by when. Two rules make it work: no service may describe the family using adjectives it cannot evidence with an observation, and the family speaks first.

Why this belongs in a week about professional identity. Because the most common cause of a family being failed by services is not any single clinician's incompetence; it is the absence of anyone whose role includes the network. In many teams, that role is nobody's. The family therapist, trained to think in systems, is often the best-placed person to take it provided they take it explicitly, rather than doing it invisibly and carrying the resentment that invisibility breeds.

5.11a Writing your first professional documents

New graduates are usually competent in the room and completely unprepared for the paperwork, which is where most complaints begin. Three documents, with the standard each should meet.

The client agreement (one page). What you offer, how long sessions last, the fee and cancellation policy, confidentiality and its limits, how records are kept and who may see them, how to reach you between sessions and what to do in an emergency, and a signature line for each adult and for the guardian of any minor. The test: a frightened person with no professional training should be able to read it once and know what to expect.

The case note (five lines per session). Date and attendees; presenting issue in the family's words; what you observed and did (including risk screening, explicitly); your formulation in one sentence; the plan and who is responsible. Written in the past tense, without speculation presented as fact, and without characterisations you would not want read aloud. The test: could a colleague who has never met this family pick up your file and continue safely?

The report (two pages). Who asked for it and what question it answers; who you saw, when, and with what consent; what you did and how; what you observed; your opinion, clearly labelled as opinion, with its limits; and recommendations that are specific, feasible and addressed to someone who can act. The test: would you be comfortable having every sentence read aloud in a hearing?

> **AUDIT**
> Take one page of any notes you have written, in a placement or a role-play, and apply the three tests above. Most students find at least one sentence that is speculation recorded as observation. Rewrite it.

5.12a The professional's first year: a survival map

The first year after qualification is the most dangerous for the practitioner and, indirectly, for clients. Five predictable pressures and the response that works.

Pressure 1 the caseload creep. You accept a referral because you can help, then another, then a family you should have referred. Response: a written maximum, reviewed quarterly, and a rule that any exception is discussed with a supervisor before it is accepted.

Pressure 2 the isolated practice. You are the only systemic clinician in your setting, so nobody can check your thinking. Response: buy or negotiate external supervision; join or start a peer group of three to five clinicians; commit to one case presentation a month.

Pressure 3 the impossible mandate. Your employer wants throughput, the family needs depth. Response: negotiate in writing what the service can and cannot deliver in the time available, and record the consequence for families (waiting lists, brief work, referral elsewhere). The record protects the families who come after yours.

Pressure 4 the case that follows you home. Response: a boundary ritual after work (a walk, a change of clothes, a specific moment of transition); a named person you can telephone; and the discipline of writing the note before you leave, which is both a legal and a psychological act.

Pressure 5 the identity crisis. Six months in, you know more about what you cannot do than about what you can, and you begin to suspect the training was inadequate. Response: this is normal and is the beginning of real competence. Track your outcomes, review recordings of your own work with a supervisor, and notice that the families are still coming back. Competence is built in the second and third year, not the first.

Week 6 The first session I: joining

6.1 The lecture in ninety minutes

This week is the assessed skills gate of the course, and it is also the week that separates students who understand family therapy from students who can do it. Joining is not a warm-up before the real work; it is the work, in its most compressed form. Everything that follows depends on whether this family will come back, and whether they will let you see what is actually happening.

The lecture has a simple structure: what joining is, why the first session is an outcome event, then four live demonstrations with the class in role.

Minutes	Segment	What the lecturer does	What students do
0 10	The evidence	Dropout figures; what predicts return	Predict which of four openings loses the family
10 25	What joining is	Minuchin's three operations: maintenance, tracking, mimesis	Mark them in a transcript

Minutes	Segment	What the lecturer does	What students do
25 40	The architecture	Pre-session, opening, contract, agenda, closing	Draft their own six-step opening
40 60	Four hard families	Reluctant father, silent adolescent, furious mother, polite family	Role-play one, in triads
60 75	Multi-alliance	Pinsof's dimensions; the balance problem	Diagnose an alliance failure on video
75 90	The gate	Rubric, what is assessed, what fails	Self-assess against the rubric

6.2 What joining is

Salvador Minuchin described joining as the therapist's effort to become part of the family system in a position from which change is possible. It has three technical operations, and students should be able to name them while doing them.

Maintenance. You affirm and support the family's existing structure and rules while you work. You do not challenge the father's authority in the first ten minutes; you do not correct the mother's description of her son; you do not require the family to reorganise itself in order to accommodate you. Maintenance sounds passive and is not: it is a deliberate choice to earn the right to disturb.

Tracking. You follow the family's own content, language and imagery rather than steering towards your agenda. If the mother talks about the kitchen, you ask about the kitchen. Tracking communicates that the family is being listened to by someone who can follow, which is rarer than students imagine and more persuasive than any technique.

Mimesis. You adopt the family's style pace, formality, volume, vocabulary, dress, seating enough to be experienced as familiar, not enough to be experienced as mockery. A loud family with a quiet therapist will either shout louder or conclude the therapist is judging them; a quiet family with a loud therapist will conclude the therapist is unsafe.

What joining is not. It is not agreement. It is not collusion with the family's account. It is not friendliness, though friendliness helps. It is not a phase you complete and leave behind: you re-join every session, and after every intervention that disturbs the system you re-join again. The rhythm of a well-conducted session is join disturb rejoin, repeated.

The test of successful joining. The family tells you something they did not plan to tell you, and they book another appointment. Those are the only two reliable indicators, and they typically occur within the first session or not at all.

6.3 Why the first session is an outcome event

Students often treat the first session as data collection and assume the real work starts later. The research does not support that model.

Dropout happens early. Across outpatient services, a substantial minority of families attend one session and never return; a larger group attends fewer than four. The single

strongest predictors of return are the early alliance and the fit between the therapist's explanation and the family's own understanding of the problem. Practical factors distance, cost, timing, childcare, whether the father can get leave matter enormously and are frequently ignored by trainees who interpret non-attendance as resistance.

The alliance is multi-dimensional. Pinsof's system for couple and family therapy distinguishes the individual alliance of each participant with the therapist, the subsystem alliance (for example, with the couple as a unit), and the whole-system alliance, across three domains: the content of the work, the goals and the tasks. A family can be high on one dimension and low on another: the mother feels understood (individual alliance high), the couple feels ganged up on (subsystem alliance low), and the family as a whole is uncertain that therapy can help (whole-system alliance low).

The practical consequence. You must monitor at least three alliances, and you cannot do it by intuition. Two habits make it possible. First, at the end of a first session, ask each person directly: "Was there anything about today that did not fit, or that you would want me to do differently?" Second, note, after the session, which member you spoke to most, which member you interrupted, and whose explanation you adopted because the recording usually contradicts your memory.

> **CAUTION**
> The most common alliance failure in the first session is not hostility; it is over-agreement with the person who speaks first. The talkative member's formulation becomes the therapist's, and the quiet member concludes that the room belongs to someone else. Count your questions by addressee after every first session for a month; the numbers will surprise you.

6.4 Preparing the session

The first session begins before the family arrives. Six decisions, each of which changes what happens.

1. What you know, and what you refuse to conclude. Read the referral; note the referring person's formulation; write down two competing hypotheses before the session, and one question that would test each. This is the discipline that prevents the referral letter from becoming your assessment.

2. Who you invite. The people in the household, plus anyone with real authority over the problem a grandmother who co-parents, an aunt who pays the fees, sometimes a pastor. The invitation should explain why: "I want to hear how it looks from everyone, because one person's view is never the whole picture." Where someone refuses to attend, ask the family how they would like that refusal handled, and consider whether the refusal is itself the central clinical fact.

3. The room. Chairs that can be moved; no desk between you and the family; tissues within reach; a clock you can see; something to write on that does not look like an assessment form. Seating matters more than students expect: a couple separated by a

child is already saying something, and where you sit either accepts that arrangement or quietly reconfigures it.

4. The interpreter. If a family needs one, use a trained interpreter, not a child, and brief them beforehand: first person, everything said, no side conversations, and the right to tell you when a phrase does not translate. See MFT 708 for the fuller protocol.

5. Your own state. Check your own distraction before the door opens. A therapist who is running late, hungry, or preoccupied with the previous session will join badly and will blame the family for it.

6. The paperwork. Consent, the practice agreement, the limits of confidentiality, and the recording consent before the clinical work, not after. Twelve minutes of housekeeping at the start is twelve minutes that protects everyone in the room.

6.5 The opening ten minutes: an architecture

There is no single correct opening, but there is a shape that reliably works, and trainees are well served by practising it until it is automatic and can then be varied deliberately.

Step 1 orient (1 minute). "I know a little from the referral, and I want to hear it from you. Before we start, is there anything you want to ask me?"

Step 2 contract (3 minutes). Who you are; what you do; how long sessions last; how many sessions you expect to need before reviewing; what happens between sessions; confidentiality and its limits, stated concretely; and what you will do in an emergency.

Step 3 agenda (2 minutes). "I would like to hear from each of you what you think is going on, and what you would want to be different. Then I will say something about how I understand it and what I would suggest. Is that all right?" The agenda is proposed, not imposed; the family may change it, and often does.

Step 4 the account (25 35 minutes). Each person in turn, with the quieter members invited explicitly and early: mothers usually speak first and children usually last, and the reverse order is often more informative. Use episode questions, not opinion questions. Notice who answers for whom.

Step 5 the sequence (10 15 minutes). Take one specific episode and slow it down until the loop is visible and the family recognises it. Do not interpret; ask until they describe it themselves.

Step 6 the feedback (10 minutes). Offer your formulation as a hypothesis, in plain language, with an explicit invitation to correct it: "Here is what I think I am seeing; tell me what I have got wrong." Then ask for one small thing to be different this week, and book the next session.

Step 7 the closing question (2 minutes). "Was there anything today that did not fit?" This is not politeness; it is alliance monitoring, and it produces information no questionnaire will.

6.5a Reading a family's style in seven minutes

Experienced therapists appear to join effortlessly because they are reading the family's style from the moment the door opens, and adjusting before they speak. The skill is teachable if you know what to look for. Seven dimensions, each with the adjustment that follows.

1. Pace. Fast, overlapping, interruptive or slow, formal, with long silences. Adjustment: match it for five minutes, then slow the fast family by about a third and hold the slow family's pace without filling their silences. A fast family experiences a slow therapist as a barrier; a slow family experiences a fast therapist as an intruder.

2. Volume and affect. Loud and expressive, or quiet and restrained. Adjustment: join at their level and come down gradually. Do not begin quieter than the family, and do not begin louder: both read as judgement.

3. Formality. Titles, dress, posture, whether humour is permitted. Adjustment: err on the formal side for the first ten minutes, then follow the family's lead. It is easy to become less formal; it is almost impossible to recover from being too familiar too early.

4. Who speaks for whom. The mother who answers for the father; the father who answers for the child; the child who translates. Adjustment: interrupt the pattern once, politely, in the first fifteen minutes, by addressing the person directly: "I would like to hear it in your words." One interruption establishes that individual voices exist; repeated interruptions establish that the therapist is at war with the family's structure.

5. The family's theory of the problem. Medical ("it is a chemical imbalance"), moral ("he is stubborn"), spiritual ("there is a curse"), systemic ("we all wind each other up"), or fatalistic ("this is just how our family is"). Adjustment: adopt the family's language for the first session while keeping your own hypothesis private. A family whose theory is spiritual will not hear a neurochemical explanation in the first hour, and a family whose theory is medical will hear a relational explanation as blame.

6. The emotional temperature of the room. Who is crying, who is frozen, who is watching everyone else, who is trying to make the therapist laugh. Adjustment: address the most distressed person first and the most watchful person second; the watchful one is usually the family's emotional barometer and will tell you, in the car park, what everyone really thinks.

7. The relationship to help. Has the family been in therapy before? Was it useful? Were they blamed? Were they reported? Adjustment: if the family has been harmed by services, name it early and explicitly: "It sounds as though professionals have not been much use to you. I would like to know what went wrong, so that I do not repeat it." Nothing joins a previously harmed family faster than an accurate account of their previous experience.

A caution about reading style. Every one of these dimensions is culturally patterned, and the therapist's own style is not neutral. If you find yourself concluding that a family is "cold", check whether you are reading restraint as absence; if you conclude a family is "chaotic", check whether you are reading a different cultural norm of overlap as disorder. The dimensions are useful for adjustment, dangerous as diagnoses.

6.6 Joining each member

Each position in the family has its own way of being joined, and the same move that joins one member can alienate another.

The mother who carries the worry. Usually the referrer, usually the most motivated, usually the person whose account has already been adopted by everyone including the school. Joining her means taking her seriously and not adopting her formulation wholesale. The sentence that works: "You have been carrying this on your own for a long time, and I want to understand how you have managed. I also want to hear how it looks from the others, because I suspect there is more going on than any one person can see."

The father who is absent or reluctant. He arrives expecting to be blamed, because he usually has been. Joining him requires making him necessary rather than merely welcome: "I cannot do this without you, because you are the only person who can tell me what happens in this house when your wife is not in the room." Ask him about his work, his own father, his view of what has gone wrong before you ask him to change anything. A father who is interviewed as an expert on his own family will often stay; a father who is interviewed as a defendant will not.

The adolescent. She has been brought, often against her will, and she has been discussed by professionals before. Joining her means giving her status and privacy: ask her permission before asking about her; offer the option of a session alone; ask what she thinks the adults have got wrong. The single most effective sentence with a reluctant adolescent is: "I am not here to make you talk about things you do not want to talk about. I would like to know what you think is going on, even if it is that this is a waste of time."

The child. Speak to the child directly, early, at their level, and about their own experience not about the parents' problem. Ask what they notice, what they would change if they had a magic wand, and what they do when the house is tense. Children are reliable reporters of sequences and unreliable reporters of feelings under interrogation; ask for the sequence.

The grandparent or kin who holds authority. Join them as an elder, not as a complication. Ask for their view of the family's history, because they usually have information that nobody else has; ask what they think the younger generation has got wrong; and negotiate their role explicitly, since a grandmother who is present but not consulted will reassert her authority at the worst possible moment.

The person who is not there. The absent member is joined through questions: "What would your husband say if he were here?" "Who would disagree with that?" The answers keep the absent member in the system and stop the present members from speaking as though the family were complete.

> **PRACTICAL**
>
> In triads, practise joining a reluctant father. The father's opening line is: "I don't know why I'm here. She's the one with the problem." You have six minutes and three moves. Then swap. Observers record how many of your questions addressed him, and whether any of them positioned him as an expert.

6.7 Handling four hard families

The furious family. They arrive angry with the referrer, with each other, with you for being the fourth professional to see them. The joining move is to name the anger and legitimise it before doing anything else: "Something has gone badly wrong for you before you got here, and I would like to know what it was." Do not defend the profession, the referrer or yourself. Give the anger somewhere to go a structured account of what happened and it usually converts into engagement within ten minutes.

The polite family. They present no problem, agree with everything, and give you nothing. Politeness is often a rule ("we do not argue in this family") rather than a personality. The joining move is to name the rule respectfully and test it: "You are all being very careful with each other. Is that how it is at home?" Then ask what would happen if someone disagreed in the room. The first disagreement is the clinical event.

The family that fights in the session. They continue the argument they were having in the car. Joining means structuring, not stopping: "I want to hear this argument, but slowly. Start again, and one of you speak at a time; I will tell you when to pause." Slowing the exchange is itself an intervention, and it converts a destructive argument into observable data.

The family in crisis. Something has just happened a death, an arrest, a disclosure, an eviction. The structure of the session changes: safety and practicalities first, formulation later. The joining move is to acknowledge the acute situation and to ask what they need in the next forty-eight hours before asking about patterns. MFT 712 covers crisis work properly; here the point is that joining in a crisis means being useful before being insightful.

6.8 The Adeyemi first session, in full

The case has appeared in previous weeks; here it is as a first session, minute by minute, with the reasoning that a trainee would not see.

Before the session. Referral from the school counsellor. Two hypotheses written down: (a) the school refusal is maintaining a caregiving role for the mother; (b) the refusal is a protest against a parental conflict that the family will not discuss. One question to test each: (a) "What does Amara do when her mother is having a bad day?" (b) "What happens between the two of you when Amara is at school?"

Minutes 0 5. Mother and Amara arrive; the father has not come. Contract stated; both agree to proceed. The therapist notes the absence and asks a single question about it "Is your husband aware you are here today?" and does not pursue it. The answer ("he knows, he had work") is recorded as data, not challenged.

Minutes 5 20. The mother's account. The therapist tracks: asks about the school, the mornings, the mother's own health, and what she has tried. She uses maintenance: "You have been doing all of this on your own." She asks Amara twice, early: "Is that how it looks to you?" Amara says "mm" and looks at her mother. The therapist notes it and does not press.

Minutes 20 35. The pivot questions from Week 2: "Before Amara stopped going to school, what was happening in the house?" and then "Who else in the family knows how bad things have been?" The mother cries. The therapist acknowledges, offers tissues, waits. Then, to Amara: "When your mother is having a bad day, what do you do?" Answer: "I stay with her."

Minutes 35 50. The sequence is slowed: what happens in the morning, who says what, what Amara does, what her mother does, what her father does when he is there. The therapist draws nothing yet; she keeps the sequence in words so that the family can hear themselves describing it.

Minutes 50 60. Feedback as hypothesis. Three sentences: what she has seen; what she thinks it might mean; what she is less sure about. Then the invitation to correct: "Have I got that wrong?" The mother corrects one detail (the father is more involved than the therapist has assumed) and the therapist thanks her and writes it down visibly, because visible correction teaches the family that the formulation is theirs to revise.

Minutes 60 70. The plan. A session with both parents, framed as a request for the father's expertise. A specific morning task, small and observable. A conversation with the school, with consent. A review in two weeks.

After the session. Notes written immediately: who attended, what was observed (two answers-for-another, one look-to-mother-before-answering), risk screening (asked about self-harm and violence; nothing disclosed), the formulation in one sentence, the plan. The therapist also writes one line for supervision: "I did not ask about the father's drinking today, and I need to decide when and how."

6.8a The Okonkwo first session, contrasted

The Adeyemi session above shows joining a family that arrives motivated and sad. The Okonkwo session shows joining a family that arrives defended, and the two together cover most first sessions a trainee will meet.

Referral. Ebuka, 9, "stubbornness". Mother attends; father does not. The mother opens with a complaint about the school, then about her son, then, in the fourth minute, about her husband, then apologises for complaining.

What is different. The mother arrives with a verdict rather than a question: the school says the child is the problem, she half-believes it, and she is braced to be told she is a bad mother. The therapist's first task is not to gather information but to lower the threat level.

Minutes 0 8. The therapist does three things before asking about Ebuka at all. She thanks the mother for coming, and mentions that coming to a clinic about a child is a decision most parents put off. She asks what the mother was told by the school in the school's words "stubborn, refuses to write" and asks what she thought when she heard it. The answer ("I thought they were saying I cannot control my own son") is the true opening of the session, and it took six minutes to reach.

Minutes 8 20. The therapist asks Ebuka, directly and first, what a good day at school looks like. He says "no writing". She asks what happens when the teacher gives him the

book; he demonstrates, physically, pushing it away. She asks what his teacher does then. He says she calls his mother. The sequence is now: worksheet push away teacher calls mother mother is angry evening reparation. The therapist has not mentioned stubbornness once.

Minutes 20 35. She asks the mother about the mornings, then about her week, then, gently, about her own support: who helps her? The answer names a sister, a neighbour, a church group, and, with a pause, "not my husband, he is working". The therapist does not pursue the husband yet. She asks: "When Ebuka is at school and writing is not happening, what is your day like?" The mother, unexpectedly, cries.

Minutes 35 50. The therapist works with the sequence rather than the diagnosis: what happens in the ten minutes before the worksheet comes out; what happens in the ten minutes after; what Ebuka does when his mother is upset. She draws nothing and interprets nothing. Twice she says, "I may be wrong about this tell me", and twice the mother corrects her, which is exactly the outcome she wants.

Minutes 50 60. Feedback. Three sentences of hypothesis, one of which is about the marriage, offered carefully and framed as a question the family can decline. Then a plan with three parts: an assessment of whether there is a writing-specific difficulty (a legitimate referral, not a dismissal of the systemic view); a morning routine the mother can run without escalation; and an invitation to the father framed as expertise.

The contrast, summarised. With the Adeyemis, joining meant widening bringing in the father, the school, the grandmother, and the marital conversation. With the Okonkwos, joining meant lowering the threat taking the verdict off the child and the mother before any widening was possible. Same skill, opposite first move. Students who learn one template and apply it to both families will fail with one of them.

6.9 Annotated transcript: joining the father who did not come

The father attends the second session. Below are the first eight turns, with the reasoning.

> **T1 Therapist:** (standing, offering a hand) Mr Adeyemi. Thank you for coming. I know it was not easy to get away.
> **T2 Father:** The office allowed me. I do not know how much I can help.
> **T3 Therapist:** You can help more than anyone else in this room, and I will tell you why in a moment. First, sit where you like. Would you rather sit next to your wife or over here?
> **T4 Father:** (sits beside his wife, slightly apart) Here is fine.
> **T5 Therapist:** Before I ask you anything about Amara, I want to ask about you. What was your week like?
> **T6 Father:** (pause) Long. The work is not regular. I take what comes.
> **T7 Therapist:** And what does that do to you, by the end of a week?
> **T8 Father:** (longer pause) It makes me tired of being asked for things.

Annotations. T1 does two things at once: it greets him by name, which recognises him as a person rather than a category ("the father"), and it acknowledges the cost of attending the first currency of joining with a reluctant man. T2 is a test: "I do not know how much

I can help" is an invitation to be told he is unnecessary. Many trainees respond with reassurance ("of course you can help!"), which is heard as hollow.

T3 does the opposite of reassurance: it asserts his necessity and defers the explanation, which creates a reason to stay. The seating question is not small talk; it is a structural intervention offered as a courtesy he chooses whether to sit with his wife, and whatever he chooses is information, given without coercion.

T5 is the crux. Asking about the child first would have confirmed the referral's frame (he is here about Amara's problem). Asking about his week establishes that he is a subject in the room, not an exhibit. T6 gives the answer that matters clinically: precarious work. T7 follows the emotion, not the fact and T8 produces the sentence that will organise the treatment: **tired of being asked for things.**

What the therapist does next, and why it matters. She says nothing for a moment, then:

> **T9 Therapist:** So you come home, and the house asks you for something too.
> **T10 Father:** (nods) Everybody wants something from me. I have nothing left to give.
> **T11 Therapist:** And when you have nothing left, what do you do?
> **T12 Father:** I sit in the car for a while before I come in. Sometimes I do not come in.

T9 is a reflection that stays with his metaphor "asked for things" becomes "the house asks you for something" which is tracking at the level of language. T11 is an episode question that produces the sequence, and T12 gives the therapist the behaviour that the family experiences as absence. No interpretation has been offered. No one has been blamed. And the father has, in four minutes, described his own side of the loop that the family has been living inside for fourteen months.

How the therapist then uses it. She does not say "so your absence is your way of coping". She turns to the mother and asks a genuine question: "Did you know about the car?" The mother did not. That single exchange conducted without accusation, in the father's presence does more for the couple's understanding than an hour of interpretation, and it is the reason the father returned for the remaining sessions.

6.10 Laboratory: the joining gate

This is the assessed skills gate for the course. Twenty minutes per student, in a triad with a trained role-player or a peer playing a scripted family, observed live and scored against the rubric below.

Task. Conduct the first twelve minutes of a first session with a family of three (two parents and a reluctant adolescent), in which the father is hostile, the mother is exhausted, and the adolescent says nothing. You must: state the contract; take an account from each person; join the hostile father; and offer one next step.

What is assessed.

Criterion	Weight	Full marks
Contract and consent	10%	Confidentiality limits and practicalities stated concretely, in plain language, checked for understanding
Agenda-setting	5%	Agenda proposed, not imposed; family's amendment accepted
Balance of speaking time	20%	Each member asked at least four substantive questions; the quietest member is not left until last
Joining the hostile member	25%	Positions him as necessary and expert; no reassurance, no defensiveness, no alliance against him
Episode questions	20%	At least three questions about a specific recent episode rather than opinions or traits
Session management	10%	Time held; interruptions managed; no attempt to cover everything
Closing and next step	10%	Hypothesis offered with an invitation to correct; one concrete next step agreed

What fails the gate, regardless of charm. Talking more than the family; adopting one member's formulation as fact; failing to state the limits of confidentiality; asking no episode questions in twelve minutes; and ignoring the silent member entirely.

Resit arrangements. A resit is available after two weeks of supervised practice, with a written reflection on what went wrong. The gate is not a punishment; it is a check that you can be trusted with a family's first hour, which is the hour that determines whether there is a second.

6.10a The feedback sequence, scripted

The last ten minutes of a first session are the most technically demanding, and the least rehearsed. Here is a script that works, with the reasoning attached.

> **1. PERMISSION.**
> "I would like to tell you what I think I am seeing. Some of it may be wrong. Will you tell me where I am wrong?" Why: it frames the formulation as a hypothesis, and it recruits the family as collaborators rather than recipients.

> **2. WHAT YOU SAW.**
> "I noticed three things. In the first half hour, Ebuka's mother answered two questions that I had asked him. When I asked about the mornings, she described shouting, and then she described him making her tea. And when I asked what happened after the teacher called, she said she felt judged by the school." Why: observations first, in plain language, each of them checkable. No adjectives.

3. WHAT YOU MAKE OF IT.
"The way I understand it is this: the writing has become the place where several worries meet Ebuka's, yours, the school's. When the school telephones, you carry the judgement alone, and by the evening the two of you have found a way to repair it. I think the repair is a strength, and I also think it is a lot of work for a nine-year-old." Why: one paragraph, no jargon, one reframe (the repair is a strength), and one gentle observation about cost.

4. WHAT YOU ARE UNSURE ABOUT.
"What I do not yet know is whether writing itself is hard for him, and whether your husband sees the same things you do. Those are the two things I would want to find out." Why: naming uncertainty is what makes the confident parts credible, and it sets up the next two sessions.

5. INVITATION TO CORRECT.
"What have I got wrong?" Why: the single most valuable question in the sequence. Families correct the therapist far more often than trainees expect, and the corrections are data.

6. THE NEXT STEP, SPECIFIC.
"I would like to see the three of you next time, and I would like to ask the school for their written record of what they have observed. Before then, I would like one thing to be different: when the school telephones, you and your husband talk first, before either of you speaks to Ebuka." Why: one task, observable, structural (it creates a parental subsystem), and achievable in a week.

7. THE SAFETY NET.
"If anything happens before we meet if anyone is hurt, or you feel you cannot cope here is how to reach me, and here is what to do if you cannot." Why: families in distress need to know the route out of the room; the therapist who omits it is assuming a stability the family may not have.

What to avoid in the feedback. Four sentences that destroy the sequence: "It is very common" (which erases the family's particularity); "You must not blame yourself" (which introduces blame); "The real problem is your marriage" (which is a verdict, not a hypothesis, and is rarely true in the simple form it is said); and "I think he may be on the spectrum" (a diagnostic opinion offered without assessment, in a first session, with no follow-up plan an error that can take months to undo).

6.10b Two opening scripts you can memorise

Trainees freeze in the first two minutes, and rehearsed language dissolves the freeze. Two scripts, to be learned and then abandoned once they are no longer needed.

Script A first session, voluntary family.

> "Thank you for coming all of you. My name is , and I am a family therapist. I work with couples and families rather than with one person at a time, because most difficulties that bring people here involve relationships rather than one person's mind. Before I ask you anything, I want to tell you four things. First, what you say here is confidential, with three exceptions: if someone is at risk of serious harm, if a child is being hurt, or if a court orders me to disclose. Second, sessions last fifty minutes. Third, I will not be able to solve this in one hour, and I do not think you expect that. Fourth, if I ask something you do not want to answer, say so and I will move on. Does that make sense? Anything you want to ask me before we start?"

Script B first session, family referred by an institution and suspicious.

> "I know that you did not choose to be here, and I know that schools or courts, or doctors can make a family feel accused. I want to be clear about my position: I am not here to judge whether you are good parents, and I have no authority over what the school decides. What I can do is help this family work out what is happening and what you want to do about it. If at the end of this hour you decide this is not useful, I will tell you honestly whether I think that is a mistake. Shall we start with what you were told, and what you think of it?"

Why these work. Both state the frame before the content; both pre-empt the family's most likely fear; both offer control ("say so and I will move on"; "if you decide this is not useful"); and neither asks a clinical question in the first minute. Script A is 170 words and takes about fifty seconds to say. Script B is 130 words. That brevity matters: a first-session monologue longer than ninety seconds makes a family anxious.

What to do when the script goes wrong. It will. A father will interrupt; a mother will cry before the contract is finished; an adolescent will ask why they are there. The rule is: the contract can be delivered in pieces, but it cannot be skipped. If you are interrupted, return to it before the clinical work: "Before we go on, I need to finish one thing, because it protects all of us."

6.11 Six errors in the first session

1. **The interrogation.** Twenty questions in a row with no reflection, no acknowledgement and no silence. Fix: one question, one answer, one brief acknowledgement; slow to two-thirds of your natural pace.
2. **The premature formulation.** A confident account of the family's dynamics in minute fifteen. Fix: no formulation before you can describe one full sequence in the family's own words.
3. **The missing safety screen.** No questions about harm to self, harm to others, violence or abuse. Fix: build the screen into the first session as a standard section, with a scripted opening that makes disclosure possible.
4. **The informal contract.** Confidentiality mentioned vaguely or not at all. Fix: a concrete script, checked for understanding, in every first session.

5. **The over-identification.** You like one member, and your questions begin building their case. Fix: count your questions by addressee; ask the quietest member first next time.
6. **The unbooked next step.** The session ends warmly and nobody agrees what happens next. Fix: end with a specific appointment, a specific task, and one sentence about what to do if things get worse before then.

6.12 Culture, faith and the first ten minutes

The joining techniques above are culturally saturated, and students should adjust them deliberately rather than applying them uniformly.

Greeting and seniority. In many contexts, the first minutes are governed by protocol: who is greeted first, whether an elder is acknowledged before the presenting problem, whether shoes are removed, whether a visitor is offered water. A therapist who does not observe these courtesies will be experienced as a stranger who does not know how to be in a home, and the clinical work will not recover from it. Ask, or observe, or ask a colleague from the community and do not treat protocol as an obstacle to the "real" work. The protocol is the work of the first ten minutes.

Language and code-switching. Families often discuss emotion in one language and practicalities in another, or switch when they are uncomfortable. Do not force a language; follow the family. If you do not speak the language they use for the most intimate material, you need an interpreter and a plan for how their words will reach you.

Naming the problem. In some families, naming a difficulty too directly in the first session is experienced as bad manners or as a curse invited. Indirect openings asking about a typical day, about what has been tried, about what the elders think will produce the information more reliably than a direct question. The rule is to take the family's route to the problem rather than yours, while making sure you arrive.

Hospitality and fees. Offer of food or drink; who pays for the session; whether a gift is appropriate. These are not trivia: they define the relationship. The professional answers are to accept hospitality graciously and briefly, to be transparent about fees before the work begins, and to have a stated policy about gifts that respects the gesture without creating obligation.

> **CAUTION**
> Do not use cultural respect as a reason to skip the safety screen. A question about violence asked with courtesy, in private, in the family's own idiom, is not a cultural insult; failing to ask it is a professional failure. Where a family's norms make a private conversation difficult, create the privacy structurally: a separate appointment, a home visit, a walk to the car.

6.12a The joined room: what good looks like from outside

Trainees rarely see a good first session from the outside, because they are always inside one. Here is a description of a session that went well, written by an observer, with the specific behaviours that made it work.

The room at minute five. The therapist has moved her chair to the corner of the family's seating rather than opposite them. The father, who arrived last and sat furthest away, has been asked where he would like to sit and has moved closer without being asked twice. The nine-year-old is drawing on paper the therapist provided, and nobody has told him to stop.

The talk. The therapist has spoken for perhaps 40% of the first ten minutes, and about 25% after that. She uses the family's words "the shouting", "the tea", "the letter" rather than clinical abstractions. Twice she has said "tell me more about that", which produced the two most informative passages of the session. In the first twenty minutes she has asked the mother six questions, the father five and the child four, each of them at least twice, and the father was asked about his work before he was asked about his son.

The moment of difficulty. At minute twenty-two the mother says, sharply, that the father does not care. The observer expects the therapist to smooth it over. Instead she slows it: "Say that to him rather than to me. Then I would like to hear what happens in you when she says it." The mother repeats it; the father is silent; the therapist waits eleven seconds, visibly, without filling the silence, and the father then says something he had not planned to say. That silence is the most skilled thing in the session.

The correction. At minute forty the therapist offers a hypothesis. The father disagrees with part of it. She writes his correction down, reads it back to him and says "that changes what I think". The observer notes the family's posture change at this point: they are working with the therapist rather than being assessed by her.

The ending. The therapist summarises in five sentences, names one task, books the next appointment while everyone is still in the room, and asks the closing question. The child's drawing is left on the table between them and the therapist asks whether she may keep it; he says yes. Nobody leaves in silence.

What the observer would teach from this session. Three things. The seating and the drawing were not decoration; they were the physical form of the message "you are welcome here and you are not on trial". The eleven-second silence is a technique of a high order, teachable only by practice. And the visible acceptance of correction turned an assessment into a collaboration, which is what the family will remember when deciding whether to come back.

> **DRILL**
> Watch any recorded first session a training video, a demonstration, or your own role-play with a stopwatch. Time the therapist's longest silence and count the times they said "tell me more". Then do the same with your own recording. The comparison is humbling and always useful.

TASK

Write the observer's account of your own last role-play in the same form: room at minute five, talk, moment of difficulty, correction, ending. Write it in the third person, as though about a stranger. Then mark the three things an observer would teach from it, and the one thing they would want changed.

6.13 Evidence note: what predicts a second session

The literature on engagement and dropout in family therapy is unusually practical, and four findings are worth knowing precisely.

The first session is the strongest single predictor. Attendance at a second session is predicted by what happened in the first alliance, clarity of the plan, and the family's sense that the therapist understood the problem. Families who leave the first session without a clear next step are much less likely to return.

Practical barriers are underestimated. Distance, transport cost, work schedules, childcare, and the timing of appointments predict attendance at least as strongly as motivation. Interventions that address logistics evening clinics, reminder calls, transport support, co-locating with schools have larger effects on attendance than most clinical techniques, and they are the least fashionable thing a service can do.

Alliance matters, and it is measurable. Session-rating measures completed by each family member after each session identify alliance ruptures early enough to repair them. The evidence supports routine use, and the practical obstacle is not the measure but the habit.

Motivation is not a fixed trait. Families described as "unmotivated" in referral letters frequently engage well when the therapist's explanation fits their own understanding of the problem. The construct that predicts engagement is not motivation in general but fit: does this explanation, in this room, make sense of what this family is living with?

The implication for the gate. The skills assessed this week are not the most sophisticated in the course; they are the ones with the best evidence for keeping families in treatment long enough for anything else to matter.

6.14 Key terms

Term	Definition
Joining	The therapist's deliberate accommodation to the family in order to work from inside the system
Maintenance	Supporting the family's existing structure and rules while working
Tracking	Following the family's content, language and imagery rather than steering to your agenda
Mimesis	Adopting the family's style, pace and vocabulary enough to be experienced as familiar
Accommodation	The therapist's adjustment to the family's system (Minuchin)

Term	Definition
Alliance (individual)	Each member's relationship with the therapist
Alliance (subsystem)	The alliance with a dyad or subsystem as a unit
Alliance (whole system)	The family's shared alliance with the therapy
Episode question	A question about a specific, recent instance of the problem
Opinion question	A question about general patterns, traits or feelings less reliable
Rupture and repair	A break in the alliance and the process of mending it
Dropout	Premature termination without agreement

6.15 Self-test

1. Name Minuchin's three joining operations and give one example of each.
2. What are the only two reliable signs that joining succeeded?
3. What are the three alliance dimensions in Pinsof's model?
4. A father says "I don't know why I'm here she's the one with the problem." What are your first two moves?
5. Why is a polite family often harder than a furious one?
6. Give the six steps of the opening architecture in order.
7. What is an episode question, and why does it outperform an opinion question?
8. What should you record after a first session that students usually omit?

Answers. 1. Maintenance (affirming the family's structure), tracking (following their content), mimesis (matching their style). 2. The family discloses something unplanned, and they book again. 3. Individual, subsystem and whole-system alliances, across content, goals and tasks. 4. Acknowledge the cost of attending and assert his necessity, then ask about his own week before asking about the child. 5. Because politeness is often a rule that forbids disagreement, so the conflict the family needs to address never appears. 6. Orient; contract; agenda; account from each person; slow the sequence; feedback and next step; closing question. 7. A question about a specific recent instance; it produces behaviour rather than characterisations and is harder to answer with a defence. 8. Risk screening, who answered for whom, and whose formulation you adopted plus what you did not ask and need to ask next time.

6.16 Reflective task

Record a twelve-minute role-play of a first session. Watch it twice: once with the sound off, noting seating, gesture, who leans forward and who looks at whom; once with sound, counting questions by addressee. Then write 300 words answering three questions. Which member did you join most easily, and why? Which member did you avoid, and what did you avoid? And what does the answer to the second question remind you of in your own family?

This is the exercise students most often skip and most need. The pattern of avoidance you find here will follow you into practice, and the only reliable remedy is to have named it early.

6.17 Readings for this week

- **Minuchin (1974)**, Families and Family Therapy, chapter 6 on joining and accommodation. Read it twice: once for the technique, once for the tone.
- **Pinsof (1995)**, Integrative Problem-Centered Therapy, on the alliance in couple and family work. The measurement framework.
- **Friedlander, Escudero & Heatherington (2008)**, Therapeutic Alliances in Couple and Family Therapy. The best single treatment of ruptures and repairs.
- **Alexander, Holtzworth-Munroe & Jameson (1994)**, on engaging resistant families. Practical and evidence-based.
- **Boyd-Franklin (2003)**, Black Families in Therapy, chapters on engagement. On the first session in families whose previous experience of services has been hostile.
- **A recorded first session with a transcript.** Any good training archive will do; read the transcript rather than watching, because reading reveals exactly what the eye edits out: the question you asked twice, the member you never addressed, the moment you filled a silence that was doing useful work.

Next: Week 7 The first session II: defining the problem

Week 7 The first session II: defining the problem

7.1 The lecture in ninety minutes

Week 6 was about joining: getting into the system in a position from which change is possible. This week is about the second half of the same hour turning a complaint into a definition of the problem that the whole family can work with. It is the most cognitively demanding skill in the first month, because it requires you to hold three things at once: the family's language, your clinical hypothesis, and the criteria by which the definition will be judged usable.

The week's central claim is simple and contested: **the way a problem is defined determines what counts as a solution.** A family that defines the problem as "he is stubborn" will accept only that he becomes compliant. A family that defines it as "the evenings have become a battle none of us knows how to end" can work on something. The therapist's job in the first session is not to impose a definition but to negotiate one that is specific, interactional, owned by the family, and changeable.

Minutes	Segment	What the lecturer does	What students do
0 10	Why definitions matter	Two definitions of one problem, and the plans that follow	Predict the plan from the definition

Minutes	Segment	What the lecturer does	What students do
10 25	The four criteria	Specific, interactional, owned, changeable tested on live examples	Score six definitions
25 45	From complaint to request	The negotiation, demonstrated twice	Negotiate with a scripted parent
45 60	Reframing the definition	From trait to sequence to function	Rewrite five complaints
60 75	Presenting problem versus underlying problem	What to leave alone in session one	Diagnose a formulation for over-reach
75 90	The agreed statement	Write it, read it back, get it agreed	Draft and test in pairs

7.2 Why the definition is the intervention

A definition of the problem is not a description; it is a set of instructions. It tells the family what to notice, who is responsible, what would count as improvement, and who has to change. Four consequences follow, and students should be able to state them.

First, a definition selects the data. A family that defines the problem as "Amara refuses school" will report on school attendance and will not report the mornings, the mother's mood or the father's drinking. The definition is a filter, and the therapist who accepts it unexamined will work with a filtered family.

Second, a definition distributes responsibility. "He is stubborn" places the problem inside a nine-year-old. "The mornings have become a battle" places it between people. The second is not kinder by accident; it is kinder because it is more accurate, and it is more useful because more people can act on it.

Third, a definition sets the criterion of success. If the problem is "he does not obey", success is obedience, and the family will be dissatisfied with anything less. If the problem is "we cannot get through a morning without shouting", success is a calm morning, which is achievable within a fortnight and which changes the family's sense of itself.

Fourth, a definition is a hypothesis about cause smuggled into a description. "She is manipulative" contains an account of intention; "she asks for her mother whenever her father enters the room" contains a sequence. The second can be tested; the first cannot. The therapist's discipline is to convert the first into the second, without correcting the family and without agreeing with them.

> **CAUTION**
> There is a difference between negotiating a definition and rewriting the family's experience. The family's account is data, and the version they bring is the version they can bear to bring. If you replace it too quickly with your own, you have not defined the problem; you have taken it.

7.3 The four criteria, tested

A usable definition of the problem meets four criteria. Teach them as a checklist and use them as a test in the room.

1. Specific. It names an occasion, a behaviour and a time frame. Not: "we have no communication." But: "on weekday evenings, after the children are in bed, we sit in different rooms and neither of us starts a conversation."

2. Interactional. It involves at least two people and at least one sequence. Not: "she is cold." But: "when I ask about her day she answers in one word, and then I stop asking, and then she says I never ask."

3. Owned. The family recognises it as theirs, in their language, without having to be persuaded. The test is simple: read it back and ask, "is that right?" If they correct you twice, the definition is still yours.

4. Changeable. It is the kind of thing that could plausibly be different within the timeframe of the work. Not: "he has always been like this." But: "the last three months, since the new job, the arguments have moved from the weekend to every night."

Scoring exercise. Six definitions, from real referral letters. Score each 0 2 on the four criteria, then rewrite the weakest.

Definition (as presented)	Specific	Interactional	Owned	Changeable	Total
"Family communication problems"	0	1	1	1	3
"Mother is over-involved with daughter"	1	0	0	1	2
"Since the father's redundancy, the parents argue about money every weekend and the daughter refuses school on Mondays"	2	2	2	2	8
"Adolescent behavioural difficulties"	0	0	1	1	2
"He will not talk to me since his brother was born"	2	2	2	2	8
"The family is enmeshed"	0	0	0	0	0

The exercise produces two lessons. Definitions from institutions score badly because they are written for categories, not families; and the best definitions sound like ordinary speech, because they are the family's own words arranged more carefully.

7.4 From complaint to request: the negotiation

Families arrive with a **complaint** (what is wrong) and often without a **request** (what they want). The therapist's task is to convert one into the other without dismissing either. Four moves, in sequence.

Move 1 get the complaint in full, in the complainant's language. Do not translate yet. "Tell me what it is like. Not the summary the last time it happened."

Move 2 find the exception or the contrast. "When was the last time it did not happen?" or "What is different on the days it is better?" This does two things: it

establishes that the pattern is not total, and it gives the family a piece of evidence about their own competence.

Move 3 ask for the wished-for change, in behaviour. "If this were better, what would I see you doing differently?" Not "how would you feel" but "what would you see". Vague answers are normal; the therapist's job is to keep asking until something observable appears.

Move 4 test the request for feasibility and ownership. "Is that something you could do this week, or is it too big?" A request that fails this test is not a request; it is a wish, and the therapist should say so gently and shrink it.

The negotiation, worked.

> **MOTHER:**
> He will not listen to me. I have to say everything five times. **Therapist:** Give me the last time. Yesterday morning? **Mother:** Yesterday. I asked him to get ready. Five times. Then I shouted. **Therapist:** And the fifth time did he get ready? **Mother:** Yes. After I shouted. **Therapist:** So shouting works. It gets him ready and it costs you the morning, and by the evening you are both tired. What would you want instead what would I see happening in that kitchen if it were better? **Mother:** I would ask once and he would move. **Therapist:** Once. And if it took twice, would that be acceptable? **Mother:** (pause) Twice would be fine.

The negotiation has produced a definition ("in this kitchen, getting ready takes five requests and a shout") and a request ("asking once or twice and he moves") that is specific, interactional, owned and changeable. Note what the therapist did not do: she did not tell the mother that shouting is bad, she did not diagnose the child, and she did not propose a technique. She found the sequence, found its cost, and let the mother name the alternative.

7.5 Reframing the definition

Reframing was introduced in Week 2 as an intervention for meaning. Here it is a tool for the definition itself. Three routes, each converting a trait into something workable.

Route 1 from trait to sequence. "He is lazy" becomes "when you ask him to start his homework, he says 'nothing', and you then do it yourself". The rewrite is not a correction; it is a request for the episode that produced the trait. Ask: "What does he do, exactly, that you would call lazy?" The answer is always a sequence.

Route 2 from sequence to function. "When you take over his homework, what happens?" "It gets done, and we stop arguing." The function is now visible: taking over ends the argument and costs the child the experience of finishing something. Naming the function must be done without blame: "the takeover is doing a job for the household."

Route 3 from function to a both-and. The mature definition holds the cost and the function: "the two of you have found a way to stop the evening becoming a fight, and it means he never has to finish anything on his own. What would let you keep the peace and let him finish?"

Five complaints, rewritten. From the case bank, with the rewrite a competent therapist would negotiate rather than impose.

Complaint	Usable definition
"My husband is controlling"	"He asks where I am three or four times an evening, and I have stopped telling him things in advance"
"My daughter is rude"	"She answers in one word and leaves the room when I ask about school, which she did not do before the exam period"
"My mother interferes"	"She telephones every morning with advice about the children, and I have not told her that it makes me feel judged"
"He drinks too much"	"He drinks four to six bottles on five nights, and on those nights he does not speak to me or the children"
"We have grown apart"	"We have not had a conversation longer than five minutes, without the children, in about a year"

The rule behind the table. Every usable definition contains a behaviour, a time frame, and at least two people. Every unusable one contains an adjective, a diagnosis or a verdict.

7.6 Presenting problem, underlying problem, and what to leave alone

The commonest error of enthusiastic trainees is to address the underlying problem in the first session. Three categories, and the discipline each requires.

The presenting problem is what the family brings. It must be taken seriously, addressed concretely, and, if possible, improved. A therapist who ignores it in favour of a richer formulation teaches the family that therapy is a place where their actual trouble is not discussed.

The underlying problem is what the therapist believes maintains the presenting one a disengaged father, an unmourned loss, a marital conflict, a family myth. It is a hypothesis, and it should be held, tested and, in most cases, not named in the first session.

The unnamed problem is the material that is not available yet: a secret, a trauma, a violence that has not been disclosed. It must be suspected carefully and not pursued beyond the safety screen. The therapist who pushes for a disclosure in the first hour often gets one that the family is not ready to have made.

A sequencing rule that serves students well. In the first session: address the presenting problem concretely, test the underlying hypothesis with two or three questions, and make one small move. Name the underlying problem only when (a) the family raises it, (b) the alliance is sufficient that a name will not feel like an accusation, and (c) you have something to offer once it is named.

CASE

A couple presents with "our son will not sleep in his own bed" (presenting). The underlying hypothesis is that the mother has not recovered from a miscarriage two years ago and the child's presence in the bed is the couple's way of not being alone together. The therapist says nothing about the miscarriage in session one. She takes a sleep history, identifies the sequence (child wakes, mother goes to him, father sleeps in the spare room), and offers a graded plan. In session four, the mother mentions the miscarriage; by then the couple have succeeded at something together and can bear the conversation. That sequencing is not avoidance; it is clinical judgement about dose.

7.7 The agreed statement

The first session ends with a negotiated statement of the problem and a first step. It should be short enough to be remembered, and it should be written down on paper, in front of the family, and given to them.

The template, in five parts.

> What happens the sequence, in the family's words, with a time frame. What it costs the consequence, stated by the family. What has been tried evidence of effort, named so it is not repeated or dismissed. What would be different the request, behavioural and small. What happens next who does what, by when.

Worked example, for the Okonkwos.

> "Over the last two months, getting ready for school has taken five requests and usually a shout, and it leaves both of you tired and upset for the day (what happens, what it costs). You have tried rewards, taking things away and talking to the school (what has been tried). What you want is to ask once, or twice, and for him to move (what would be different). Next week I would like to see all three of you, and before then, when the school telephones, I would like the two of you to talk to each other before either of you speaks to Ebuka (what happens next)."

Reading it back. The statement is read aloud and offered for correction. Two corrections are typical: the family will want to add something they consider important, and they will want to soften a word that sounds like an accusation. Accept both; the statement is better with them.

Giving it to them. A written copy on a single sheet the statement, the task, the next appointment is one of the cheapest and most effective interventions available. Families keep these; they show them to relatives; they remind themselves of what they agreed when the week gets difficult.

7.7a The Adeyemi definition, negotiated in real time

The general method becomes concrete in a single case. Here is the negotiation, as it happened, with the therapist's reasoning.

The complaint as offered. "Amara refuses to go to school. She has missed six weeks. The school is threatening to report us." Three complaints in two sentences: a behaviour, a duration and an institutional threat. The therapist records all three and returns to the first.

The first attempt at specificity. Therapist: "Tell me about this morning. Not the whole six weeks this morning, from waking up." Mother: "I woke her at six. She said she was not going. I said she had to. She pulled the cover over her head. I shouted. She cried. I left her." Therapist: "And then what did you do?" Mother: "I telephoned the school and said she was sick."

What the therapist now has. A sequence with five links, a lie told to an institution, and a mother who is managing alone. Note that the sequence is not "Amara refuses school"; it is "waking, refusal, escalation, tears, and a telephone call that converts a family difficulty into an absence". The definition is already more useful than the complaint.

The second attempt the cost. Therapist: "What does that morning do to the rest of your day?" Mother: "I am angry all day. And then I feel guilty. And then I wait for her to come out of her room." The cost is now specific and is the mother's own, not the institution's.

The third attempt the request. Therapist: "If I could change one thing about that morning, what would you want it to be?" Mother: "That I did not have to lie." This is the pivot of the session, and it is not what the therapist expected. The mother does not ask for attendance; she asks to stop deceiving the school. The therapist follows it: "So the lie is the worst part." Mother: "Because they will find out and then they will think I am a bad mother."

The reframe. Therapist: "So you have been protecting your daughter from the school's judgement, and yourself from being seen as a bad mother, at the cost of being alone with something you cannot manage." Mother: (cries) "Yes."

The negotiated statement, written on paper and read back.

> "For six weeks, most mornings have gone: you wake Amara, she refuses, you insist, she cries, you telephone the school and say she is ill. The cost is that you spend the day angry and guilty, and you are carrying something alone that you believe will make people think you are a bad mother. You have tried insisting, talking, and threatening. What you want first is to stop having to lie. What I suggest is that we tell the school the truth together a letter from me, and a conversation with you present and that we do not make attendance the first goal. The first goal is that you are not alone with this."

What makes this statement good. It is specific (six weeks, most mornings, five links); interactional (mother and daughter, then mother and school); owned (the mother's words: "I did not have to lie", "a bad mother"); and changeable (a letter and a conversation can happen this week). It also does something that a formulation based on the referral could never have done: it identifies the institutional loop as part of the problem, and it makes

the therapist an ally in the mother's relationship with the school rather than an agent of the school's demand.

The lesson for trainees. The definition you would have written from the referral letter ("school refusal in a 16-year-old, six weeks") is accurate and unusable. The definition the family negotiated is accurate and actionable, and it took fourteen minutes of questions and one reframe.

7.8 Annotated transcript: negotiating with a family that does not agree

In many first sessions the family does not have one problem; it has three. The negotiation of a shared definition is the clinical work. Below is a twelve-turn excerpt from a family in which the parents' definitions are incompatible.

> **T1 Mother:** The problem is his temper. He shouts at the children and then they come to me.
> **T2 Father:** The problem is that she undermines me. I say no, she says yes, they learn that I mean nothing.
> **T3 Therapist:** So there are two accounts. Let me hold both for a moment. Mother, when he shouts, what do the children do?
> **T4 Mother:** They cry and come to me. Then I tell him he is frightening them.
> **T5 Therapist:** And then, Father?
> **T6 Father:** Then I leave the room. There is no point.
> **T7 Therapist:** So: he shouts, they go to her, she tells him he is frightening them, he leaves. Is that what happens most weekend evenings?
> **T8 Both:** (nods) Yes.
> **T9 Therapist:** Which part of that would each of you most want to be different? Not the whole thing one part.
> **T10 Mother:** I would want him not to shout.
> **T11 Father:** I would want her to back me up in front of them, even if she disagrees with me.
> **T12 Therapist:** Those are different requests, and they might be connected. Let me see whether I can put them together in a way that is honest for both of you.

Annotations. T3 is the pivot, and it is a refusal to arbitrate. The therapist does not decide whose definition is correct; she says she is holding both, which is both truthful and the only position from which the loop can be described. Students who arbitrate even silently, by asking more questions of one partner lose the other.

T3 T8 build a sequence that both partners confirm. Note the technique: the therapist asks each partner about the other's behaviour, not their own, which is easier to answer honestly and produces the loop rather than two monologues. The confirmation at T8 ("is that what happens most weekend evenings?") matters: a definition the family has confirmed is a definition they cannot easily disown later.

T9 is the request-elicitation move, and the constraint ("not the whole thing one part") is doing important work. It prevents the session from becoming a negotiation of everything, and it produces two requests that can be compared. T10 and T11 are incompatible as

stated he must not shout; she must back him up when he shouts which is precisely the material for the next several sessions.

T12 names the incompatibility honestly and promises an integrative move without pretending there is none. The therapist's next sentences, not shown, construct a both-and: "What would let him speak firmly without shouting, and let her support the decision without supporting the shouting?" That question converts two competing requests into one joint task.

The most common trainee error in this transcript. Asking "how do you feel when he shouts?" That question is sympathetic, and it makes the loop harder to see, because it invites each partner to describe their own inner state rather than their joint behaviour. Feelings become clinically useful after the sequence is visible, not before.

7.9 Laboratory: the definition negotiation

Ninety minutes. Triads, with a scripted family played by two peers from a case card, and an observer scoring against the rubric. Each student takes the therapist's chair once, for twenty minutes.

The card. Both parents present, one adolescent present and silent, and a referral letter containing a diagnosis. The parents' definitions conflict. The adolescent has not been asked.

The task. Produce, in twenty minutes: a sequence the family confirms; one behavioural request from each parent; a both-and formulation that does not require either parent to concede; and a written statement of five lines, read back and agreed.

Marking rubric.

Criterion	Weight	Full marks
Sequence elicited and confirmed	25%	A behavioural loop, stated in the family's words, confirmed by both partners
Requests elicited	20%	One behavioural request per parent; vague requests converted into observable ones
Non-arbitration	20%	Both definitions held; no silent siding; questions balanced
Integrative formulation	20%	A both-and that addresses both requests without collapsing them into one
Written statement	15%	Five lines; specific, interactional, owned, changeable; read back and corrected

The pass standard. Four of five criteria at 3/5 or better, with sequence and non-arbitration at 4/5. Students who fail most often fail on non-arbitration, and the usual cause is a sympathetic identification with the parent who arrived first or who is more articulate.

7.9a The goals grid: four columns, one page

Families rarely have one goal; they have several, held by different members, of different sizes, with different time horizons. A single page, completed in the first or second session, prevents most of the confusion that follows.

Person	What they want (behavioural)	By when	What they would accept as a first step
Mother	To stop lying to the school; to have one calm morning a week	Six weeks	A letter to the school this week
Father (when present)	To know what is happening in the house; to stop being the last to hear	Six weeks	A fortnightly conversation with his wife before the session
Amara	To be left alone in the mornings; not to be asked about school every day	Two weeks	Nobody mentions school before 9 a.m.
Therapist	A shared definition the family owns; one structural change in the parental subsystem	Four sessions	Both parents in the room next week

How to use the grid. Complete it in front of the family, reading each entry back. Where a row is blank, say so: "Amara, I do not have anything from you yet." Where two entries conflict, name the conflict as the work: "Your mother wants one calm morning and you want no conversation before nine. Those might be the same thing."

Why a grid rather than a conversation. Because the conversation produces the illusion of agreement, and the grid produces evidence of disagreement. The families who benefit most from therapy are often those whose members want incompatible things, and the grid makes that incompatibility discussable without making anybody the villain.

> **TASK**
> Complete a goals grid for a family you know from placement, role-play or the case bank. Bring it to the seminar. Then answer: whose goal did you find hardest to write in behavioural terms, and why? The answer is usually the goal you understand least, which is the goal you will accidentally neglect in the work.

7.10 Six errors in defining the problem

1. **Accepting the referral's definition.** The letter says "oppositional behaviour" and the session proceeds as though that were established. Fix: ask the family what they would call it, and note the difference.
2. **The premature insight.** Naming the underlying problem in the first session, before the family has the resources to hear it. Fix: test the hypothesis by questions, name it only when the family raises it.
3. **The vague request.** Accepting "I want us to communicate better" as a goal. Fix: ask what you would see, on a Tuesday, that would tell you it had improved.
4. **The single request.** Agreeing only the more articulate partner's goal. Fix: ask each person separately and write both down, even if they conflict especially if they conflict.

5. **The definition that blames.** A statement the family experiences as an accusation, however carefully worded. Fix: read it back and ask for corrections; if a member goes quiet, the definition is not agreed.
6. **The missing safety clause.** A definition that omits risk violence, self-harm, safeguarding because the family did not raise it. Fix: the screen comes before the definition, always.

7.10a Rewriting six institutional definitions

Referral letters are written for institutions. This week's most practical skill is translating them without losing the information they contain.

1. "Poor engagement; family disengaged from services." Translation: the family has attended four services, each of which asked them to do something they could not do, and the fourth asked them to come to a clinic two buses away on a weekday morning. What to do: ask what each previous service asked of them, and change the practical conditions before interpreting the motivation.

2. "Mother is over-involved; enmeshed relationship with daughter." Translation: the mother is the only adult who has been managing a chronic difficulty, and the daughter has become her confidante because the marriage is disengaged. What to do: widen the support, not detach the daughter; interview the father.

3. "Non-compliant with treatment plan." Translation: the plan required something the family could not sustain daily homework, weekly attendance, a medication schedule that conflicts with shift work. What to do: ask what happened at home when they tried; redesign the plan around the family's actual week.

4. "Adolescent is manipulative; splits parents." Translation: the adolescent has learned that approaching one parent produces a different answer from the other, and uses the difference to get what most adolescents want, which is to be allowed out. What to do: work with the parents on a consistent position; do not diagnose the adolescent.

5. "Child has behavioural difficulties at school only." Translation: the difficulty is a property of the school child fit, and possibly of the classroom, not of the family. What to do: obtain the school's observations before assuming a family pattern; involve the school in the plan.

6. "Family in denial." Translation: the family does not accept the institution's definition of the problem, and has reasons. What to do: ask what they think the problem is, and what they make of the institution's account. "Denial" is usually a description of a disagreement, written from the side that has more power.

The general rule. Every institutional definition contains information about the family and information about the institution. Your job is to separate the two, and to notice that the second is often more revealing than the first.

7.10b The definition audit: five questions after every first session

A checklist to complete in the ten minutes after a first session, before the notes. It takes five minutes and it improves the second session more than any technique.

1. Whose words are in my definition? Count the phrases in your agreed statement that came from the family, and the phrases that came from you or from the referral. A statement with more of your language than theirs is a formulation, not an agreement, and it will not survive a difficult week.

2. Who did not contribute a goal? Every family has a member who says nothing when goals are requested. Name them in your notes, and plan how to hear from them next time usually alone, usually first, and usually with a question about what they would change if they had a magic wand.

3. Did I define the problem in a way that requires someone to become a different person? "He needs to be less aggressive" is a definition that requires a personality change. "When he raises his voice, his wife leaves the room and takes an hour to return" is a definition that describes a pattern two people can alter.

4. What would count as evidence, next week, that the definition was wrong? If nothing could count, you have a belief rather than a hypothesis. Write the evidence condition in your note.

5. Did I leave the session with a statement I could read aloud to the family without flinching? If not, the statement contains something you would not defend in their presence, which means it contains something you have not yet said to them. That is the material for session two.

7.11 Culture, faith and the definition of a problem

The definition of the problem is one of the most culturally loaded moments in the session, because it determines what kind of difficulty this is: medical, moral, spiritual, relational or social. Four adjustments.

Whose problem is it? In many families, the person who telephones is not the person with authority to define the problem. A mother may describe the difficulty, but the father, the grandmother or an uncle may hold the right to name it. A definition agreed with the wrong person will not hold. Ask, early: who else would need to agree that this is the problem?

The spiritual definition. "This is a spiritual attack" is a complete definition, with its own aetiology, treatment and criterion of success. It competes with yours and should not be dismissed. The usable move is to establish what the family would count as evidence of improvement, and to negotiate a definition that allows both accounts: "The family believes this is spiritual, and we are also going to look at what happens in the household, because both matter for a child." Note that where a spiritual definition leads to a child being denied treatment, the professional obligation overrides the negotiation.

The moral definition. "He is disobedient" is a moral claim, and in many families it is a claim about the parents' standing as much as about the child. Challenge it directly and you challenge the parents. Instead, keep the moral frame and widen it: "Disobedience is

serious in this family. I want to understand what makes it possible in this house, because that is what we can change." The parents' values are preserved; the target moves.

The social definition. Poverty, migration status, housing and unemployment are definitions of the problem that therapists often translate into psychological language to their cost. A family whose problem is that the father has no work does not need a formulation about boundaries. Ask the practical question first, and let the family tell you whether the difficulty is material, relational or both.

> **CAUTION**
> Do not use "we need to understand the family's meaning system" as an excuse for not stating the obvious. If a child is not attending school, or a woman is being hit, the professional's definition of the problem has to include it, in plain language, whatever the family's preferred account.

7.11a Working with a family whose definition is a diagnosis

Increasingly, families arrive with a diagnosis already in hand from a school, a paediatrician, a previous clinician, or increasingly from the internet. The diagnosis is both a definition and a resource, and it must be handled with respect rather than either deference or dismissal.

What a diagnosis gives a family. A name, which reduces chaos; an explanation, which reduces self-blame; an entitlement to services; a community of other families; and, often, a moral status ("this is an illness, not bad behaviour"). These are substantial benefits, and a therapist who attacks the diagnosis attacks the family's coping.

What a diagnosis costs. It can locate the problem entirely inside one person, which removes the family's leverage; it can freeze a developing picture into a label; it can become the family's only way of talking about the child; and it can license a fatalism ("this is how he is") that forecloses change.

The systemic move. Do not dispute the diagnosis; widen the definition around it. Four sentences that work:

> "The diagnosis explains a lot, and it does not explain everything. In this house, the difficult hours are between six and eight in the evening is that right? Let us look at what happens in those hours, because those are the hours we can change, whatever the diagnosis says about the rest."

The family keeps its explanation and acquires a target. The therapist does not have to win an argument about aetiology in order to do useful work.

A caution about diagnostic enthusiasm. The opposite error is equally common: a family arrives with a relational story, and the clinician supplies a diagnosis that reorganises the family's understanding of their child. The responsibility that comes with naming a condition is significant, and it belongs to an assessment process, not to a first session (see MFT 707).

7.12 Evidence note: goal consensus and outcome

Three findings from the literature on goals and definition of the problem, all practically useful.

Goal consensus predicts outcome. Across psychotherapy research, agreement on goals and tasks between client and therapist is one of the more robust predictors of outcome, and disagreement about goals is a common precursor to dropout. In family work the finding is stronger and more complicated, because there are several clients: consensus must be built with each member, and where members' goals conflict, the therapy must accommodate the conflict explicitly rather than resolve it by fiat.

Behavioural specificity improves engagement. Families given vague goals report less progress and discontinue more often than families given specific, observable goals. This is not merely a measurement convenience; a specific goal allows a family to notice change, and noticing change is what sustains hope in the early weeks.

The first-session formulation predicts attendance at session two. Families who leave the first session with a formulation they recognise in their own words, offered as a hypothesis are significantly more likely to return. The mechanism is probably the experience of being understood, which is the same construct the alliance literature measures.

The implication. The negotiating skills in this week are not a preliminary to treatment; they are among its active ingredients. A therapist who spends the first session chasing information and leaves without an agreed statement has spent the hour on the least evidence-based part of the work.

A fourth finding, on measurement. When families and therapists independently rate the goals of therapy, their agreement is moderate at best, and the discrepancy is not random: therapists tend to rate goals as more shared than families do. The practical lesson is to ask rather than to assume, and to ask in a form that can be answered concretely "which of these three would you put first?" rather than "do we agree on the goals?" The question that invites a yes tells you almost nothing, and the question that offers a choice tells you which of the family's three priorities they will actually act on in the following week.

7.12a Two definitions, two plans: the same family, a year apart

A comparison that makes the week's argument vivid. The same household, defined two ways, six months apart, by two different clinicians.

Definition A (a community clinic, first appointment after the referral). "Amara, 16, school refusal for six weeks, with anxiety features; mother reports exhaustion; father works away." The plan that follows: an anxiety assessment for Amara, a graded return-to-school plan negotiated with the school, individual CBT, and a review in six weeks. What happened: Amara attended two sessions, the graded plan collapsed in the second week, and the family did not return.

Definition B (a family therapist, six months later). "The mornings have become a battle in which the mother asks five times, shouts, and then telephones the school with an

explanation she knows is not true; the cost is a day of anger and guilt for her, and a daughter who is at home and cannot leave; the father is outside this sequence and has not been asked what he thinks." The plan that follows: a letter to the school, written with the mother; both parents in the room; a small morning task; a conversation about the marriage when the parents are ready; review in two weeks. What happened: the family attended eleven sessions; the school return was staged over two months; the parents' arguments became discussable; the father did not stop drinking, and the household still improved.

What the comparison is not saying. It is not saying that CBT is wrong, that anxiety was not present, or that the first clinician was incompetent. It is saying that the first definition located the problem where nothing in the household could be changed, and the second located it where several things could. Two accurate descriptions, one usable.

The question for the examination. "What is the relationship between the accuracy of a formulation and its usefulness?" A strong answer says that they are different virtues: a formulation can be accurate and inert, or actionable and incomplete, and the professional's task is to be accurate and to choose, deliberately, the level of description at which change is possible.

7.13 Key terms

Term	Definition
Presenting problem	The difficulty the family brings and wants addressed
Underlying problem	The therapist's hypothesis about what maintains the presenting problem
Complaint	What is wrong, as described by the complainant
Request	The behavioural change the family wants
Goal consensus	Agreement between therapist and clients on goals (and on tasks)
Reframing	Offering a new frame for the same facts that supports a different response
Both-and formulation	A statement holding two apparently incompatible truths without collapsing them
Agreed statement	A short written summary of the problem, its cost, the request and the next step
Exception	An occasion when the problem did not occur, or occurred less
Scaling question	A question asking the family to place the problem or their coping on a numbered scale

7.14 Self-test

1. What are the four criteria of a usable definition of the problem?
2. Why is "he is stubborn" a poor definition? Give two reasons.
3. What is the difference between a complaint and a request?
4. Give the four moves of the negotiation from complaint to request.
5. What are the three routes of reframing a definition?
6. Why should the underlying problem usually not be named in the first session?

7. What are the five parts of the agreed statement?
8. What does the research say about goal consensus?

Answers. 1. Specific, interactional, owned, changeable. 2. It names a trait rather than a behaviour, so it cannot be observed or tested; and it places the problem inside one person, so only that person can change. 3. A complaint describes what is wrong; a request describes the behaviour the family wants instead. 4. Take the complaint in full; find the exception or contrast; ask for the wished-for change in behaviour; test for feasibility and ownership. 5. Trait to sequence; sequence to function; function to a both-and. 6. Because it is a hypothesis, and naming it before the family has the resources to hear it produces defence or compliance rather than collaboration. 7. What happens; what it costs; what has been tried; what would be different; what happens next. 8. Goal consensus predicts outcome, and disagreement about goals precedes dropout.

7.15 Reflective task

Take a difficulty you have described about your own family, in the words you actually use "my mother is controlling", "my brother is irresponsible", "we are not a demonstrative family". Now run it through the three reframing routes: the sequence it hides, the function it performs, and the both-and that holds both truths. Write 300 words on what you had to give up to write the both-and. Most students find they had to give up a satisfying certainty.

7.16 Readings for this week

- **Minuchin (1974)**, Families and Family Therapy, chapter 7, on problem definition and restructuring.
- **de Shazer (1985)**, Keys to Solution in Brief Therapy, on well-formed goals. Read it alongside the family therapy literature, because it is the clearest writing on specificity.
- **Friedlander, Escudero & Heatherington (2008)**, chapters on goals and on ruptures the empirical case for this week's skills.
- **Weakland, Fisch, Watzlawick & Bodin (1974)**, "Brief therapy: focused problem resolution", Family Process. The classic statement on negotiating a workable definition.
- **Anderson & Goolishian (1988)**, "Human systems as linguistic systems", Family Process. The opposite view problem-dissolving rather than problem-solving worth arguing with.
- **Any current clinical guideline** for the presenting problem in your placement, to see how institutions define problems and what that definition does to the family's own account. Read one guideline as if you were the family receiving it: what does it say the problem is, who does it say is responsible, and what would it count as failure? That exercise takes twenty minutes and changes how you write a formulation for the rest of your career.

Next: Week 8 The genogram as a hypothesis machine

7.10b The definition audit: five questions after every first session

A checklist to complete in the ten minutes after a first session, before the notes. It takes five minutes and it improves the second session more than any technique.

1. Whose words are in my definition? Count the phrases in your agreed statement that came from the family, and the phrases that came from you or from the referral. A statement with more of your language than theirs is a formulation, not an agreement, and it will not survive a difficult week.

2. Who did not contribute a goal? Every family has a member who says nothing when goals are requested. Name them in your notes, and plan how to hear from them next time usually alone, usually first, and usually with a question about what they would change if they had a magic wand.

3. Did I define the problem in a way that requires someone to become a different person? "He needs to be less aggressive" is a definition that requires a personality change. "When he raises his voice, his wife leaves the room and takes an hour to return" is a definition that describes a pattern two people can alter.

4. What would count as evidence, next week, that the definition was wrong? If nothing could count, you have a belief rather than a hypothesis. Write the evidence condition in your note.

5. Did I leave the session with a statement I could read aloud to the family without flinching? If not, the statement contains something you would not defend in their presence, which means it contains something you have not yet said to them. That is the material for session two.

7.11a Working with a family whose definition is a diagnosis

Increasingly, families arrive with a diagnosis already in hand from a school, a paediatrician, a previous clinician, or increasingly from the internet. The diagnosis is both a definition and a resource, and it must be handled with respect rather than either deference or dismissal.

What a diagnosis gives a family. A name, which reduces chaos; an explanation, which reduces self-blame; an entitlement to services; a community of other families; and, often, a moral status ("this is an illness, not bad behaviour"). These are substantial benefits, and a therapist who attacks the diagnosis attacks the family's coping.

What a diagnosis costs. It can locate the problem entirely inside one person, which removes the family's leverage; it can freeze a developing picture into a label; it can become the family's only way of talking about the child; and it can license a fatalism ("this is how he is") that forecloses change.

The systemic move. Do not dispute the diagnosis; widen the definition around it. Four sentences that work:

> "The diagnosis explains a lot, and it does not explain everything. In this house, the difficult hours are between six and eight in the evening is that right? Let us look at what happens in those hours, because those are the hours we can change, whatever the diagnosis says about the rest."

The family keeps its explanation and acquires a target. The therapist does not have to win an argument about aetiology in order to do useful work.

A caution about diagnostic enthusiasm. The opposite error is equally common: a family arrives with a relational story, and the clinician supplies a diagnosis that reorganises the family's understanding of their child. The responsibility that comes with naming a condition is significant, and it belongs to an assessment process, not to a first session (see MFT 707).

7.12a Two definitions, two plans: the same family, a year apart

A comparison that makes the week's argument vivid. The same household, defined two ways, six months apart, by two different clinicians.

Definition A (a community clinic, first appointment after the referral). "Amara, 16, school refusal for six weeks, with anxiety features; mother reports exhaustion; father works away." The plan that follows: an anxiety assessment for Amara, a graded return-to-school plan negotiated with the school, individual CBT, and a review in six weeks. What happened: Amara attended two sessions, the graded plan collapsed in the second week, and the family did not return.

Definition B (a family therapist, six months later). "The mornings have become a battle in which the mother asks five times, shouts, and then telephones the school with an explanation she knows is not true; the cost is a day of anger and guilt for her, and a daughter who is at home and cannot leave; the father is outside this sequence and has not been asked what he thinks." The plan that follows: a letter to the school, written with the mother; both parents in the room; a small morning task; a conversation about the marriage when the parents are ready; review in two weeks. What happened: the family attended eleven sessions; the school return was staged over two months; the parents' arguments became discussable; the father did not stop drinking, and the household still improved.

What the comparison is not saying. It is not saying that CBT is wrong, that anxiety was not present, or that the first clinician was incompetent. It is saying that the first definition located the problem where nothing in the household could be changed, and the second located it where several things could. Two accurate descriptions, one usable.

The question for the examination. "What is the relationship between the accuracy of a formulation and its usefulness?" A strong answer says that they are different virtues: a formulation can be accurate and inert, or actionable and incomplete, and the professional's task is to be accurate and to choose, deliberately, the level of description at which change is possible.

Week 8 The genogram as a hypothesis machine

8.1 The lecture in ninety minutes

The genogram is taught badly almost everywhere, as a data-collection instrument: symbols to memorise, information to gather, a diagram to file. That version of the genogram is tedious for the family and useless for the clinician. The version in this week is a hypothesis machine a device for making visible what nobody in the family has said aloud, and for generating questions that could not otherwise be asked.

The technique is associated with Murray Bowen, who used family diagrams in the 1960s as part of his multigenerational approach, and with Monica McGoldrick and Randy Gerson, whose Genograms in Family Assessment (1985) standardised the symbols and, more importantly, the interviewing. The standardisation is a convenience; the interviewing is the method.

Minutes	Segment	What the lecturer does	What students do
0 12	What a genogram is for	Three functions: structure, repetition, relationship	Predict what each function reveals in a case
12 25	Symbols and conventions	Draw live, deliberately including errors	Copy, then criticise the drawing
25 45	The interview	Demonstrate the standard sequence on a volunteer	Interview a peer about their family of origin
45 60	Reading the map	Six patterns that recur across generations	Identify patterns in two maps
60 72	The date-line and the year	Why dates are the most powerful questions	Calculate two family dates
72 85	Genogram with the family present	Demonstrate the shared drawing	Draw with a role-played family
85 90	Set the assignment	The three-generation map and its commentary	Plan their own genogram

8.2 What a genogram is for

Three functions, and a student should be able to say which one they are using in any given session.

Function 1 structure. The genogram makes visible who is in the family, who is not, who is close, who is cut off, who has died, who has left, who is spoken about and who is never mentioned. A family that describes itself as "just the four of us" will produce, in twenty minutes of genogram work, an absent father, two deceased grandparents, a stillborn child and a cousin who lived with them for three years. The structure is not merely background; it is the field in which the presenting problem is located.

Function 2 repetition. The genogram makes visible patterns across generations: the age at which people leave home, marry, die, or become ill; the recurrence of a particular kind of loss; the repetition of a role (the caretaker, the black sheep, the one who never

married). Repetition is the most clinically powerful thing a genogram reveals, because it converts a personal difficulty into a family pattern, which is both less shaming and more actionable.

Function 3 relationship. The genogram records the quality of relationships: close, distant, conflicted, cut off, abusive, fused. These are the family's own descriptions, drawn as they are described, and the drawing makes the emotional geography available for comment in a way that speech does not. "You have drawn your mother very close and your father at the edge; is that how it feels?" is a question a diagram makes askable.

What the genogram is not. It is not an assessment instrument with a scoring system (whatever the commercial packages suggest); it is not a substitute for asking about the present; and it is not a test the family can fail. Its output is not a diagram but a set of hypotheses and a set of questions.

8.2a What the map cannot show

A technique is only understood when its limits are. Five things a genogram cannot tell you.

It cannot show the family's emotional life. A double line means "close" as the family describes it; it does not tell you whether the closeness is warm, anxious, controlling or comforting. The map records the family's description, not the family's experience, and the two diverge exactly where the clinical interest lies.

It cannot show what is happening now. A genogram is a history. The date-line brings it as far as the present, but the sequence in the kitchen on Tuesday morning is not on the map, and that sequence is where the work is done.

It cannot show strength of evidence. A line drawn from one member's account is drawn in the same ink as a line confirmed by four. Mark your sources, or you will forget which is which when you reread the file in six months.

It cannot show the family's own theory. Families have explanations spiritual, moral, medical, relational and the genogram has no notation for them. Record them in words beside the diagram: "Family's account: this began after the grandmother's burial, and the family believes the burial was not done properly."

It cannot show the network. Schools, clinics, churches, mosques, employers, immigration authorities and courts are all part of the field in which the family lives. A genogram that stops at the household boundary will systematically miss the institution that is actually keeping the problem going (see Week 9).

8.3 Symbols, conventions and the discipline of drawing

The standard conventions are simple and worth learning precisely, because imprecision is read by the family as carelessness.

Element	Convention
Male	Square

Element	Convention
Female	Circle
Gender unknown or not disclosed	Diamond
Deceased	Symbol with a cross, with date of death
Marriage / partnership	Horizontal line between the symbols, with the date above
Divorce	Two slashes across the marriage line, with the date
Separation	One slash
Cohabitation	Dashed horizontal line
Children	Vertical line from the marriage line, then siblings in birth order left to right
Twins	Line from a single point, then branching
Pregnancy loss / stillbirth	Small filled circle at the end of a short line
Adoption	Dashed vertical line to the child
Foster child	Dashed line with a small "F"
Household membership	Dotted rectangle around those living together
Close relationship	Double line
Conflict	Zigzag line
Distant	Dashed line
Cutoff	Line with a single slash through it
Abuse	Double line with a zigzag, or a labelled arrow (use with care)

Five drawing disciplines.

1. **Draw big.** A genogram on a postcard is a diagram of the therapist's impatience. Use a full sheet.
2. **Three generations minimum.** Two is a family tree; three is a pattern.
3. **Dates on everything.** Births, deaths, marriages, divorces, migrations, the onset of the problem. The dates are where the clinical gold is.
4. **Label what you infer.** If you draw a conflict line because of something the family said rather than explicitly, mark it with a question mark. The family will correct you, which is the point.
5. **Let the family hold the pen.** At least once in the work, ask a family member to draw or correct the map. The corrections are the assessment.

8.4 The interview: a standard sequence

The genogram interview has a shape, and following it prevents the two common failures: gathering endless facts and never asking about feelings, or asking about feelings and never establishing the facts.

Step 1 the household, now. Who lives here? Ages, occupations, health, who sleeps where, who is at school or working. Five minutes. Draw as you go, and say what you are drawing.

Step 2 the parents' families of origin. For each parent: their parents (names, ages, occupations, health, deaths), their siblings, and the quality of those relationships. This is where most of the material is, and it is where trainees spend the least time.

Step 3 the events timeline. Deaths, separations, migrations, losses, illnesses, job changes, moves with dates, placed against the family's history. Then the crucial question: what was happening in the family when each of these occurred?

Step 4 the presenting problem on the timeline. When did it start? What else was happening then? The answer frequently identifies the event that the family has not connected to the problem, and sometimes the event that nobody has mentioned at all.

Step 5 the relationship questions. Who was closest to whom? Who did not speak? Who was the one everyone worried about? Who was the one nobody worried about? Who is most like whom?

Step 6 the questions the map makes possible. "Your father left home at seventeen; your son is seventeen." "Your mother died in March; the school refusal began in April." "You are the eldest of six and your daughter is the eldest of two." Ask the question, then wait. The family's answer including "that is a coincidence" is the clinical material.

Timing. A full three-generation genogram takes 40 60 minutes with a cooperative family, and it is rarely possible in a first session. Two approaches work: a partial genogram in session one (household and the parents' families, drawn quickly and deliberately unfinished), completed over the next two sessions; or the genogram as the agenda for session two, announced at the end of session one. Never rush it; a rushed genogram is a form.

8.4a When a member will not take part

Somebody always refuses the genogram. Usually it is the father, sometimes the adolescent, occasionally the grandmother who "does not see what the past has to do with it". Refusal is information, and it is manageable.

The three refusals, and what they mean.

The unprepared refusal. "I do not see the point." This is the most common and the least meaningful. It is a comment about the therapist's explanation, not about the technique. Fix: explain the purpose in one sentence and give a small, low-threat task: "I am going to ask your wife about her family for ten minutes. If you would correct me when I get something wrong, that would help."

The protective refusal. The member does not want certain material discussed in front of certain people a death, a previous marriage, a child given away, a prison sentence. This is almost always a request for a different format, not a refusal of the work. Fix: offer the genogram in a different configuration. "Would you rather we did this with just the two of

us first, and then I bring it back to the family?" Almost every protective refusal becomes cooperation once the audience changes.

The defensive refusal. The member has understood that the genogram will make their own history visible, and does not want it examined. This is the refusal to respect and to work with slowly. Fix: do not insist, do not interpret the refusal in the room, and note it: a family member who cannot bear to look at three generations is often the person carrying the most.

What not to do. Do not proceed as though the refuser were absent, drawing their side with a shrug and a comment about "the gaps". Families hear these comments. Do not make the refusal the topic of the session either the therapist who spends twenty minutes processing why the father will not discuss his mother has made the father the problem.

The long game. The genogram can be returned to at any point in the therapy, and the member who refused in session two often volunteers the crucial piece of information in session eight, when something else has been resolved.

8.5 Reading the map: six patterns that recur

The genogram's value is in pattern recognition, and patterns are learned by looking at many maps. Six recur constantly in clinical work.

1. The repetition of an age. A father whose own father died when he was eleven becomes anxious when his son turns eleven. A woman who left home at seventeen becomes distressed when her daughter turns seventeen. The age is the trigger, and nobody has connected the two events because they are forty years apart.

2. The repetition of a loss at the same point in the life cycle. Miscarriage, stillbirth, the death of a child, a parent's death during the respondent's adolescence. Families often organise themselves around an unmourned loss, and the organisation is visible in the map: a gap in the birth order, a name that is never mentioned, a child who is described as "the one who came after".

3. The replacement child. A child conceived or named after a dead sibling, or treated as though they were the dead person. The pattern is visible in the map when a birth follows a death within a year, and in the interview when the child is described in terms that belong to someone else.

4. The cut-off. A branch of the family with whom contact has ceased, often after a dispute whose original subject nobody remembers. Cut-offs are transmitted: children of cut-off parents often have no relationship with half their kin, and the map makes the amputation visible.

5. The role that recurs. The caretaker, the black sheep, the one who never married, the one who "held the family together". These roles are frequently occupied by one person per generation, and the map shows who is next in line.

6. The migration pattern. Who left, who stayed, who was left behind, and at what cost. In families with significant migration, the map carries an emotional geography as

important as the relational one: the grandmother who raised the children while the parents worked abroad, the sibling who was left and never forgave, the money that flowed one way and the obligation that flowed back.

> **PRACTICAL**
> Take any published genogram from a textbook and read it in ninety seconds, writing one hypothesis and one question. Then read the case discussion. Comparing your hypothesis with the author's is the fastest way to learn pattern recognition, and it takes ten minutes per case.

8.6 The date-line: the most powerful question in the technique

If a student learns one thing from this week, it should be the use of dates.

Why dates work. Families live in time, and they rarely place events against each other. A mother who says "my father died when I was young" and "my son has always been difficult" has not noticed that "always" began in the March her father died. The date-line does the noticing for her, and it does it visibly, on paper, in front of everyone.

How to use it. Draw a horizontal line beneath the genogram, mark the years, and place the events the family has mentioned. Then place the onset of the presenting problem. Then simply point at the proximity and ask an open question: "What was happening in the family in that year?" Nothing more. The family will do the work.

What to place on the line. Births, deaths, marriages, divorces, migrations, job losses, house moves, illnesses, accidents, imprisonments, the onset of the presenting problem, and the date the family first sought help. The last one is often revealing: a family that seeks help five years after the onset has usually had a reason to wait.

Three examples of what the date-line produces.

A couple present with sexual difficulties that began "about two years ago". The date-line shows that the wife's mother died two years and one month ago, and that the couple moved into the mother's house six weeks after the death. Nobody had connected the two.

A father presents with anger at his 14-year-old son. The date-line shows that the father was 14 when his own father left the family, and that the son's difficulties began in the month the father was promoted to a job requiring travel. Two repetitions at once.

A woman presents with panic attacks. The date-line shows a stillbirth eleven years earlier, never discussed, and the panic began in the month her eldest daughter the child born after the stillbirth turned eleven.

The rule. Use dates, draw them, and ask the open question. Do not interpret the coincidence for the family; point at it and let them speak.

8.6a Three date-line cases from the case bank

The technique is best learned by applying it. Three short cases, each with the events as the family first gave them and the placement that changed the question.

Case 1 Folake and Dele. Presented problem: "we argue about everything, mostly about money, for about a year". Events as first given: married 2016; first child 2018; Dele's business incorporated 2021; Folake returned to full-time work 2023; a car accident in 2023; the arguments began "about a year ago". On the date-line the therapist placed the accident and the return to work in the same month, and asked what happened after the accident. Folake had been unable to drive for four months; Dele had driven her everywhere; the arguments began when she resumed driving and no longer needed him. The question "what changed when you got the car back?" produced the session's central material, and it had nothing to do with money.

Case 2 Mrs Okonkwo and Ebuka. Presented problem: "he will not listen; the school says he is defiant". Events: Ebuka born 2016; father posted to another state 2022; grandmother died 2023; a new baby 2024; the school's first letter 2024. The date-line showed that the defiance began in the term after the grandmother's death, and that the grandmother had been Ebuka's daytime carer from 2017 to 2022. The therapist's question was not about discipline but about who looked after him when his father left; the mother wept, and the "defiance" acquired a bereavement.

Case 3 Aisha and Sam. Presented problem: "we cannot talk without it becoming a fight since we married". Events: met 2019; married 2021; Aisha's father died 2022; Sam's job loss 2023; a miscarriage 2024, never mentioned by either partner in the session until the date-line. Placed on the line, the miscarriage sat three months before the couple's first appointment and eleven months after the job loss. The therapist said only: "There is a date here that we have not talked about." That sentence, and the twenty minutes that followed, was the treatment.

The pattern across the three cases. In each, the family's "about a year ago" or "always" concealed a specific date, and the specific date was connected to a loss that had not been spoken about. This is not because loss explains everything; it is because families often present the argument and not the bereavement, and the date-line is the cheapest way to invite the second into the room.

8.7 Drawing with the family present

The genogram is most powerful when the family watches it being drawn, because the drawing itself becomes an intervention.

The mechanics. Large sheet, on a table the family can see, or on a flip chart. Draw slowly. Say what you are drawing as you draw it, and use the family's words. Ask for confirmation at each step. Leave gaps deliberately where you do not know something, and mark them with a question mark.

What happens in the room. Three things, reliably. First, members correct each other, and the corrections reveal disagreements about family history that nobody knew existed ("your father did not leave in 1998, he left in 1996"). Second, members contribute information that the identified patient has never heard, because the conversation is addressed to the map rather than to the child. Third, the family's emotional temperature often drops, because the task is concrete and shared, which makes the genogram useful with families that cannot tolerate direct emotional work early on.

The question that does the most work. After the map is drawn, ask: **"Who is missing from this picture?"** Families produce, in response, the stillborn child, the sibling who died in infancy, the father who left, the aunt who is never mentioned, the child given up for adoption. The answer is the most clinically valuable sentence of the session.

The second question. "What would change on this drawing if things got better?" This converts the map from a history into a plan, and it gives the therapist the family's own criteria of change.

> **CAUTION**
> Drawing a genogram in front of a family can surface material that is painful and unexpected: a death that some members did not know about, or a secret that slips out. Be prepared to stop, to acknowledge, and to ask whether the family wants to continue. The technique must never override the family's pace.

8.7a Mapping three household structures

The standard genogram assumes a nuclear household. Three common Nigerian structures require deliberate adaptation, and each is best taught with a worked description.

Structure 1 the extended household with an absent parent.

A household of seven: a grandmother (68), her daughter (34, the mother), three children (14, 11, 6), a niece (19) and a nephew (16) whose own mother lives in Abuja. The father works in Port Harcourt and returns monthly.

Standard drawing. Father and mother with a marriage line, three children below, and the niece and nephew attached to nobody, floating in the corner of the page.

The drawing that matches the family. The father is drawn at the top right, in a dotted rectangle labelled "Port Harcourt, monthly"; a cohabitation-style dashed line to the mother, labelled "monthly"; the grandmother's square drawn close to the mother's circle with a double line; a dotted rectangle around grandmother, mother, the three children, the niece and the nephew, labelled "same compound"; the niece and nephew attached to their own mother in Abuja by a dashed line, labelled "fostered since 2019".

Why the second drawing is better. It shows at a glance what the first hides: that the meaningful parental structure in this household has three adults in it, one of whom is physically absent; that the grandmother is the daily authority; and that two children in the house are there because of an arrangement that has a date. The first drawing produces the hypothesis "father is disengaged". The second produces the hypothesis "the parental subsystem is a grandmother mother coalition, with a father who participates monthly and two children whose placement has never been discussed". The second is more useful and more accurate.

Structure 2 two households, one father.

A father (41) with a wife (38) and two children (12, 8) in Ibadan, and a second partner (33) with one child (5) in Abeokuta. All four adults know each other. The second partner's child spends school holidays in Ibadan.

The drawing. Two unions from the father's square, each with its own line a marriage line to the wife, a partner line to the second partner the children attached to the correct line, and a dotted rectangle around the Ibadan household, with an arrow from the Abeokuta child into the rectangle labelled "holidays".

What the map makes discussable. The obligations, the money, the school fees, the question of who is introduced to whom, and frequently the wife's experience of the arrangement, which she has never been asked about in a clinical setting. Drawing the family accurately is itself an act of respect; drawing it as though the second household did not exist tells the family that the therapist has already decided what counts as a family.

Structure 3 the transnational family.

Parents migrated to the United Kingdom in 2015; three children remained in Lagos with the paternal grandmother, and the eldest joined the parents in 2021 at the age of fifteen; the two younger children joined in 2023.

The drawing. The parents' squares and circles in a dotted rectangle labelled "UK since 2015"; the children connected, with a migration arrow and date beside each movement; the grandmother's symbol in a separate dotted rectangle labelled "Lagos", with a double line to each of the children, labelled "raised her".

What it reveals. The sibling group has been separated and reunited in a particular order, at particular ages, and the person who did the raising is not in the household. The eldest child, who joined first at fifteen, spent six years as an only child in a new country; the youngest, who joined at twelve, spent her early adolescence with her grandmother and joined her parents at the point when adolescents typically begin to separate. These are facts about attachment, obligation and belonging, and they are visible in ninety seconds on the right drawing.

The general principle. Draw what is there. Every adaptation to the standard symbol set is justified if it makes the family more recognisable to itself, and every departure from the standard should be explained to the family in one sentence: "I am drawing it this way because this is how you have described it to me."

8.8 Annotated transcript: the genogram that changed the session

A sixth-turn sequence from a second session with the Adeyemi family. The mother has agreed to a genogram; the father is present but has said little; Amara is at school.

> **T1 Therapist:** I would like to draw your two families your parents, your brothers and sisters, and the dates. It takes about twenty minutes and I will ask you to correct me. Mother, your side first. Your mother's name?
> **T2 Mother:** Mama. She is 71. She lives in Ibadan.
> **T3 Therapist:** (draws) 71. And your father?
> **T4 Mother:** He died. 1994. I was 19.
> **T5 Therapist:** (draws, then pauses) You were 19. And you are the eldest?
> **T6 Mother:** I am the eldest of five. When he died I stopped school and I looked after them. My mother was not she was not well after it.
> **T7 Therapist:** You looked after four children at 19, while your mother was unwell. (draws) Where were you all?
> **T8 Mother:** In the same house. Until I married. I married at 24.
> **T9 Therapist:** (to Father) And your side?
> **T10 Father:** My father is 68, he is in Lagos. My mother died in 1998. I am the third of six.
> **T11 Therapist:** (draws) How old were you in 1998?
> **T12 Father:** Sixteen. I left for Lagos that year. For work.
> **T13 Therapist:** And your mother had died that year.

(The therapist draws a date-line: 1994, 1998, 2001, 2008, 2019, 2025. She places the deaths, the mother's marriage, the father's departure, the births of the children, and then the onset of the school refusal.)

> **T14 Therapist:** Here is your mother's father's death in 1994. Here is your father's mother's death in 1998, and your leaving in the same year. Here is 2008, when Amara was born, and 2019, when your husband lost the job. And here is this year, when the school refusal began.
> **T15 Mother:** (after a pause) 2019 is when everything changed.
> **T16 Therapist:** What happened in 2019?
> **T17 Mother:** He was at home for two years. Then he found this work, but it is in Lagos. He is away Monday to Friday. (pause) I am alone with them, like I was alone with my sisters.
> **T18 Therapist:** You were 19 and you were looking after four children, and now you are looking after three children alone on weekdays. (pause) Nobody has said that out loud in this room.
> **T19 Father:** She has not said it to me either.
> **T20 Therapist:** (draws a line between the parents, marked "distant, weekdays") Is that the right line?

Annotations. T1 announces the task, its duration, and the correction clause. The correction clause is what makes the technique collaborative rather than extractive, and it produces better data.

T5 is the date-line question asked early, and it does more work than any question about the marriage. T7 repeats the mother's sentence back, which is a simple and under-used technique: repetition without interpretation invites elaboration.

T13 is a one-sentence observation, followed by silence. Note that it is not an interpretation. It is a statement of fact, in sequence, which the mother can take or leave.

T14 T17 is the date-line doing the clinical work. The therapist has not said that the mother is repeating her adolescence, or that the father is repeating his own departure. She has placed the events on a line, and the mother has done the rest.

T18 is the most dangerous sentence in the transcript and it is carefully built: it names a repetition, but it names it as an observation about the mother's life rather than as an accusation about the father. The sentence is delivered with two facts, not one interpretation, and it is followed by a pause long enough for the mother to respond.

T19 is the moment the session changes. The father speaks, and what he says ("She has not said it to me either") is both a defence and an opening. The therapist does not pursue the defence; she draws a line, labels it with the family's own description, and asks for correction. The line is the intervention: the distance is now on the paper, where both of them can see it, rather than in the mother's private resentment.

What the therapist avoided. She did not ask the father why he drinks. She did not connect his mother's death to his absence, although the connection is obvious and may be right. She did not use the word "abandonment". Each of these would have closed the session in a way that this sequence opens it.

8.9 Laboratory: the three-generation genogram

The assessed practical for this week runs over two sessions: one in class, one completed at home.

Part A (in class, 60 minutes). In pairs, take a full three-generation genogram of your partner's family of origin, using the standard sequence. Then swap, and take the genogram of the partner's chosen family the people they would call family, which is often not the people on the birth certificate. The second version is the harder and more instructive task.

Part B (at home). Draw your own three-generation genogram on a single A3 sheet, with dates, and write a 500-word commentary answering four questions: what pattern repeats; what event coincides with the onset of a difficulty in the family; who is missing from the map; and what the map suggests you would find hardest to ask about.

Marking rubric.

Criterion	Weight	Full marks
Symbolic accuracy	20%	All symbols correct; dates present; household marked
Coverage	20%	Three generations; both sides; losses, cut-offs and migrations included
Interviewing	20%	Standard sequence followed; family's words used; corrections invited
Pattern reading	25%	Two defensible hypotheses, each tied to a specific feature of the map
Commentary	15%	Honest; identifies the student's own blind spot without exhibitionism

Two warnings for the assignment. Students sometimes produce a genogram that contains material a family member would not want shared. The rule is that the sheet stays in the student's possession and only the commentary is submitted, with names replaced.

And students sometimes discover something about their own family in the exercise that they need to talk about. That is not a failure of the assignment; it is the assignment working, and there is supervision available.

8.9a Reading a published genogram: an exercise in hypothesis generation

The skill this week is not drawing; it is generating hypotheses and knowing which ones to test. Here is the exercise that develops it fastest, done in pairs in fifteen minutes.

The map. Take any published genogram with a case discussion McGoldrick and Gerson contains dozens. Read only the diagram and the three-line referral. Do not read the discussion.

Step 1 three structural facts. Write the three most striking structural features: an unusual age gap, a death at a particular point, a cut-off, a household composition that does not match the birth order, a cluster of events in one year. Keep to facts on the page.

Step 2 two hypotheses. Write two competing hypotheses that would explain the referral, each tied to a different structural fact. The discipline of two is important: it prevents premature certainty and it trains the habit of holding alternatives.

Step 3 the discriminating question. For each hypothesis, write the single question whose answer would most reduce your uncertainty. This is the most important step, and the one students skip.

Step 4 read the discussion. Compare. The published author will usually have seen one of your hypotheses and missed the other, and will sometimes have seen something you did not. Note which.

Worked example. A referral: "Parents request help with a 9-year-old son, defiant at home." The map shows a father who is the eldest of five, his own father dead at 62 of a stroke when the father was 9; a mother who is the youngest of three, her parents alive; the son is the couple's only child; the father's youngest brother is drawn with a conflict line to the father.

Structural facts: the father was 9 when his own father died; the son is 9; the father is in conflict with the brother who is closest in age to the son; the son is an only child, in a family where the father was one of five.

Hypothesis 1 (the age repetition): the son's ninth year has reactivated the father's own loss, and his anxiety is expressed as strictness, which the son experiences as defiance.

Hypothesis 2 (the role): the father's conflict with his youngest brother has moved into the household, and the son has become the brother he cannot speak to.

Discriminating questions: "What was your father like when you were nine?" (tests H1). "When you argue with your son, who does he remind you of?" (tests H2). Either answer is informative, and both are askable in the first session.

What the exercise teaches. The genogram does not give answers; it gives the shape of the question. A clinician who leaves the map with three structural facts, two hypotheses

and two questions is ready for a second session. A clinician who leaves with a diagram has drawn a picture.

8.10 Six errors in genogram work

1. **The questionnaire genogram.** Twenty minutes of questions with no drawing in front of the family, and no hypothesis. Fix: draw, say what you are drawing, and ask the sequence questions aloud.
2. **Two generations.** A map that stops at the parents cannot reveal repetition. Fix: three generations, always, even if the third is sparse.
3. **No dates.** A genogram without dates is a diagram of names. Fix: dates on births, deaths, marriages, moves and the onset of the problem.
4. **The therapist's interpretation delivered as fact.** "So your mother's death is why you cannot let your daughter go." Fix: place the events, ask the open question, and let the family make the connection or not.
5. **Ignoring the missing.** A map that reproduces only what the family offers will omit the disowned, the dead and the never-mentioned. Fix: ask "who is missing?" and draw what appears, including the gaps.
6. **The genogram as an end in itself.** Six sessions of history and no present-day work. Fix: the genogram is a hypothesis machine; every hypothesis it produces should generate a question about this week.

8.10a The ten-minute genogram

Most clinical settings do not permit a sixty-minute genogram interview. A workable compressed version takes ten minutes and produces two hypotheses, which is more than most first sessions produce.

Minutes 1 3: the household. Who lives here, ages, and who else is involved in raising these children. Draw as you ask; keep it rough.

Minutes 4 6: two dates. The year the parents married, and the year the presenting problem began. Ask what else was happening in the year the problem began.

Minutes 7 8: one question about the parents' own childhoods. "When you were your son's age, what was your household like?" Ask both parents; draw a rough sketch of each family of origin.

Minutes 9 10: one question about the missing. "Who else is in the family who we have not mentioned?" Draw what appears. Stop.

The output. Two hypotheses and two questions for the next session, in the notes. Nothing more. The compressed version is not a substitute for the full interview, and it should be completed in the second or third session when the material justifies it.

8.11 Culture, faith and the genogram

The genogram looks like a neutral technique and is not. Five adjustments for work in Nigeria and in any context where kinship does not follow the nuclear template.

Kinship is wider than the diagram. In many Nigerian families, the household includes cousins, wards, nieces, apprentices, house-help and elderly relatives, and obligations extend to a wider lineage than the genogram's three generations can show. A genogram that records only the parents and their children will systematically miss the people who are raising the children and the people who hold authority over them. Draw the household as it actually is, with a dotted line around everyone who eats from the same pot, and ask about the wider lineage separately.

Polygyny and multiple households. A genogram based on a single marriage per person misrepresents a family with two households, co-wives, or children raised between homes. The convention is to draw each union honestly, in whatever form it takes, with a cohabitation line, a marriage line or a partner line, and to record where each child was raised. The map must match the family, not the template.

Names and the dead. Some families will not speak the names of the dead, and some will not discuss a death at all. The genogram must be built at the family's pace, and the therapist must be able to leave a symbol undrawn. Ask permission: "Some families do not like to say the names of those who have died. Tell me what is comfortable for you, and I will draw only that."

The spiritual lineage. In families with strong religious or traditional commitments, the significant "ancestors" are not only biological. A map that includes the church, the mosque, the imam, the pastor, the traditional healer or the family's deity as significant figures in the family's structure is often more accurate than a purely biological one. Ask: "Who else is a parent in this family in the sense of having authority or responsibility for these children?"

Migration and remittance. A genogram of a transnational family should record who migrated, when, and who was left behind with whom. The grandmother who raised four grandchildren while their parents worked abroad is a structural fact of the family, not a footnote, and the guilt, obligation and resentment that accompany the pattern are frequently the material of the presenting problem.

> **DEBATE**
> Is the standard genogram a culturally specific instrument that imposes a Western kinship model on families whose structure it cannot represent? Take a position, and argue it with reference to a family you know.

8.12 Evidence note: what the genogram is and is not

The genogram's evidence base is thinner than its popularity suggests, and honesty about this is part of professional competence.

What is established. Genograms are reliable as descriptive instruments: trained clinicians can produce similar maps from the same family, and inter-rater agreement on structural facts (household composition, deaths, marriages) is generally good. Genogram data have been used in family medicine to identify risk patterns, in genetic counselling to map heritable conditions, and in systemic research as a way of recording family structure.

What is not established. There is little evidence that genogram-derived interpretations the multigenerational hypothesis, the "repetition compulsion" reading, the assumption that a pattern in the map explains a symptom in the present are valid. The genogram generates hypotheses; it does not confirm them. A therapist who treats the map as an explanation has confused a heuristic with a finding.

What the research does support. The genogram is a powerful engagement device. Families report the genogram interview as one of the more comfortable and revealing parts of an assessment, and the technique reliably produces information that families have not previously connected. Its value is in the process the drawing, the corrections, the date-line rather than in the diagram that results.

The professional implication. Use the genogram to ask better questions, not to reach conclusions. Record the family's own account of the pattern alongside your hypothesis, and mark your hypothesis as such. When you write a report, distinguish what was drawn from what was inferred, and give the family the opportunity to correct the inference before it becomes part of a record that will outlast the therapy.

8.12a Putting the genogram in the record

The genogram will be filed, and the filing requires judgement. Four rules.

Record the family's account and your hypothesis separately. The record might read: "Family's account: father's mother died 1998, when he was 16; he left for Lagos that year. My hypothesis: father's own experience of loss at 16 may be connected to his difficulty in remaining present for his son's adolescence; not yet tested." The distinction matters for supervision, for any subsequent clinician, and for the family if they ever read the file.

Do not record unconfirmed third-party information as fact. If a mother says that her husband's sister was "sent away" at 17, and nobody confirms what happened, the record says "mother reports" not "aunt was sent away".

Obtain agreement about who sees the map. The genogram is a document about people who are not in the room, and those people have not consented to being mapped. In a shared record, mark the diagram as clinical material with restricted access where the system allows it, and never circulate it beyond the treating team without consent.

Give the family their own copy, if they want one. Some families will want to keep the map, to take it home and correct it. This is a good outcome: the map that the family corrects at home is better than the map the therapist files.

Storage. A genogram contains health information about relatives who are not clients. Follow the same rules you would for any clinical document: secure storage, defined retention period, no photographic copies on personal devices, and no use in teaching without consent and anonymisation thorough enough that the family could not recognise itself.

8.12b Two genograms of one family, and the difference it made

The final worked example for the week, showing how the drawing changes the formulation.

The map produced in session two. Mother, father, two children aged 9 and 4. Father's parents: both alive; father is the second of four. Mother's parents: both alive; mother is an only child. Presented problem: the 9-year-old's tantrums.

The formulation that followed. A firstborn child, displaced by the arrival of a sibling, in a family where the mother has no experience of siblings and the father has three. The tantrums were read as a protest about the dethronement. The plan was a behavioural programme for the tantrums and a discussion of the parents' expectations of a firstborn.

The map produced in session five, after the family had corrected it. The father is the second of four, but the first of those four died at 14 months, before the father was born. The mother is an only child whose own mother had four pregnancies and three losses. Nobody had mentioned either fact, because in both families the losses were "not spoken about".

The formulation that followed. Two parents who had each grown up in a household organised around a child who died, and who had each learned that a child's survival was not to be assumed. The 9-year-old's tantrums were being met not with ordinary irritation but with alarm, and the father's nightly checking of the child's breathing — which the family had not mentioned either — now made sense as a symptom of the parental history rather than of the child's behaviour. The tantrums were addressed, and so, in the following weeks, was the checking.

Why the father's checking was not asked about. Because the therapist had a map that made it unnecessary to ask. The tantrums were the referral, the parents were cooperative, and the programme was working well enough to satisfy the school, the parents and the therapist. A formulation can be adequate to the referral and still miss the thing the family most needs to say, and the only reliable protection against that is to keep asking the structural questions after the crisis has passed, when there is time to hear the answers.

The lesson. The first map was accurate as far as it went. It lacked two symbols and a date, and those two symbols reorganised the case. The therapist's job is not to draw the perfect map in the first session; it is to keep the map open, to keep asking who is missing, and to expect that the answer will change the case a second time — as this one did in the fifth session, when the family said the breathing had never been mentioned because there was nothing to say.

8.13 Key terms

Term	Definition
Genogram	A multigenerational family diagram with dates, relationships and events
Date-line	A horizontal timeline on which family events and the onset of the problem are placed
Cut-off	A relationship in which contact has ceased, often transmitted across generations
Replacement child	A child conceived or treated as a substitute for a deceased sibling
Fusion	A relationship in which the two parties have little separate identity (Bowen)

Term	Definition
Differentiation	The capacity to remain oneself while emotionally close to others (Bowen)
Triangle	A three-person configuration in which tension between two is displaced onto a third
Family of origin	The family in which a person was raised
Family of procreation	The family a person forms as an adult
Horizontal stressor	A difficulty occurring in the present generation
Vertical stressor	A difficulty transmitted from previous generations (McGoldrick)
Household marker	A dotted line around those living together

8.14 Self-test

1. What are the three functions of a genogram?
2. Why is three generations the minimum?
3. What is the date-line, and what question follows it?
4. Name four patterns that recur across generations and are visible on a genogram.
5. What is the most useful question to ask once the map is drawn?
6. What does the evidence say about genogram interpretation?
7. How should a genogram represent a family with two households?
8. Why should the family hold the pen at least once?

Answers. 1. Structure, repetition, relationship. 2. Two generations show a family; three show a pattern, because repetition requires comparison across at least three. 3. A timeline of family events with the onset of the problem placed on it; the question is "what was happening in the family in that year?" 4. Repetition of an age; repetition of a loss at the same life-cycle point; the replacement child; the cut-off; the recurring role; the migration pattern. 5. "Who is missing from this picture?" 6. Descriptive reliability is reasonable; interpretive validity is not established the genogram generates hypotheses rather than confirming them. 7. Draw each union honestly, with its own line type, and record where each child was raised. 8. Because the corrections are the assessment, and drawing distributes ownership of the map.

8.15 Reflective task

Draw your own genogram before the seminar, with dates, and place on the date-line the year you decided to train as a therapist. Then write 250 words on the proximity of that decision to any other event on the line. Students frequently find that the decision is closer to a family event than they had realised; that discovery is not a diagnosis of your motives, it is an example of how the technique works.

8.16 Readings for this week

- **McGoldrick, Gerson & Petry (2008)**, Genograms: Assessment and Intervention, 3rd edition. The standard text; read the chapters on the genogram interview rather than the symbol list.
- **Bowen (1978)**, Family Therapy in Clinical Practice, especially the chapters on multigenerational transmission and triangles.
- **McGoldrick & Gerson (1985)**, the first edition, for the historical development of the conventions.
- **Kerr & Bowen (1988)**, Family Evaluation, chapters 4 6, for the theory the genogram was designed to serve; read chapter 5 twice if triangles are still abstract, because they will not be after the second reading, and then draw three of your own from memory before you go back to the book.
- **Bennett & Wolin**, any paper on the "resilience genogram", for a strengths-oriented variation.
- **A published case study** that includes a full genogram and a date-line, read to see how an experienced clinician moves from the drawing to a hypothesis.

Next: Week 9 Formulating relationally

Week 9 Formulating relationally

9.1 The lecture in ninety minutes

Assessment gathers. Formulation explains. The distinction is not academic: a service can be full of assessments and empty of formulations, and the result is a file that records everything and understands nothing. This week is about the second half of the process turning the material of Weeks 6 to 8, plus whatever standardised measures the service uses, into a relational explanation that generates a plan and can be tested.

The week's central claim: **a formulation is a hypothesis written in a form that could be wrong.** Everything else follows from that sentence. If your formulation could not be wrong, it is a description or a belief. If it could be wrong but you have not said what would show it to be wrong, it is not yet usable. If it could be wrong and you have said how, you have a professional document.

Minutes	Segment	What the lecturer does	What students do
0 10	Assessment versus formulation	Two files on one case, both complete	Identify which explains
10 25	What a formulation does	The five jobs, with examples of each	Match jobs to sentences from real reports
25 45	A relational template	Build one on the board from a case	Supply the material from the case bank
45 60	Competing formulations	Four lenses on one family; four plans	Choose a lens and defend the plan

Minutes	Segment	What the lecturer does	What students do
60 72	Writing it	The one-page format; the testability clause	Draft one page in pairs
72 85	Presenting it to the family	Demonstration and correction	Rehearse the first three sentences
85 90	The network	Who else is in the system, and who decides	Map one real network

9.2 What a formulation is, and the five jobs it does

A formulation is a provisional, collaborative explanation of why this difficulty, in this family, at this time, and what would change it. Five jobs, each of which a good formulation performs explicitly.

Job 1 it organises the information. The assessment produces forty facts: ages, diagnoses, dates, scores, incidents, accounts. A formulation selects and arranges them so that some are central and others are background. The selection is a judgement and should be visible as one: "I am treating the father's absence as central, and the school's response as background, because the pattern changes when he is at home."

Job 2 it explains the maintenance. The most important single sentence in any formulation is the one that answers: what keeps this going now? Not why it started that is usually unknowable and almost never the lever but what is holding it in place this month. A formulation without a maintenance account is a history.

Job 3 it predicts. A formulation should say what will happen if nothing changes, and what should change first if the intervention works. "If the parents continue to disagree about discipline in front of the child, the child's behaviour will not change, whatever programme is used; if they agree on one rule for two weeks, the behaviour should reduce within a fortnight." That sentence is testable, and it commits the therapist.

Job 4 it generates a plan. The plan should follow from the formulation with a visibility that a colleague could trace. If the formulation says the problem is maintained by the parents' disagreement, the plan must include work on the parents' disagreement; a plan of individual sessions for the child would contradict the formulation, and either the plan or the formulation is wrong.

Job 5 it can be revised. Formulations are hypotheses, and they change with evidence. A formulation that survives twenty sessions unchanged is either miraculously correct or no longer being used.

What a formulation is not. It is not a diagnosis (a classification of a condition), not a summary (a condensed record), not an interpretation delivered to the family as insight, and not a story that makes the therapist feel clever. The temptation of the elegant formulation is the subject of one of this week's errors.

> **CAUTION**
> Formulation is a professional activity with a real risk of harm: a persuasive formulation can create a self-fulfilling account that the family adopts and cannot challenge. Write formulations in language the family could read, and give them the chance to disagree with it.

9.2a Bringing the numbers into the formulation

Most services now use standardised measures, and trainees often treat them as an audit requirement rather than as clinical material. Three ways to build self-report data into a relational formulation.

Use the discrepancy, not the score. The SCORE-15 and similar family measures give several viewpoints: each member rates the family's communication, the difficulty, and their own coping. The total score is the least interesting number on the page. What matters is the pattern of disagreement: a family where the mother rates the difficulty at 5 and the father at 1, when both are describing the same household, is telling you something about who carries the worry. The formulation should say so: "The parents' ratings of the difficulty differ by four points on a five-point scale; the mother's rating is consistent with her account of being alone with it."

Use the change, not the level. A family that begins at a high level of distress and falls sharply is improving faster than a family that begins at a moderate level and does not move. Plot the scores, show the family the plot, and ask them what they make of it. Families find their own data persuasive in a way that a therapist's interpretation is not, and the plot gives the therapist a shared object of attention that is not a person.

Use the feedback to break the bad-news loop. Where the scores for a young person are far worse than the parents' ratings, the therapist has evidence for something the young person may not be able to say and the parents may not be able to hear. The formulation can hold it: "The two ratings do not match, and both are true this is how it looks from where each of you is sitting." This is often the most useful intervention of a first session.

What measures cannot do. They cannot tell you the mechanism, they are culturally loaded in ways that are still poorly understood, and they are easy to over-interpret. A score is a sentence the family has said in a constrained format; treat it as one piece of what the family has said, not as the family's verdict.

9.3 The 4 Ps in a systemic key

The most widely used formulation structure in clinical training is the four Ps: predisposing, precipitating, perpetuating, protective. It transfers well to family work if each P is rewritten in relational terms.

Predisposing (relational history). What in the family's history makes this difficulty more likely? Patterns transmitted across generations, attachment experiences, previous losses, models of marriage and parenting, migration history, experiences of

discrimination, and the family's relationship with institutions. Not "genetics" alone, and not a list of traumas: an account of how the family learned to manage the kind of stress it now faces.

Precipitating (the event and its meaning). What happened, and what did the family make of it? The event is usually identifiable a redundancy, a death, a move, a diagnosis, an adolescence, a birth. The meaning is what converts an event into a crisis: "his redundancy meant that he was no longer the provider, and in this family that is what a man is." Both parts are needed; the event alone is a chronology, the meaning alone is a mood.

Perpetuating (the pattern). What is keeping it going now? The cycle between the members, the family's attempted solutions, the responses of the wider system (school, employer, clinic), and the meanings that have hardened. This is the section that does the clinical work, and in a family formulation it should name at least one loop with three or more elements, and at least one attempted solution that has become part of the problem.

Protective (the resources). What is holding the family together and what can be built on? Not a list of compliments: a specific inventory of capacities who works, who is affectionate with whom, what the family has survived, which relationships are intact, which professionals are allies, what the family does well on good days.

A fifth P for family work: the pattern of help-seeking. How has this family used help before, and what happened? Families that arrive with a history of failed services have a formulation-relevant history of failed services, and the history predicts how this episode will go unless the therapist changes something (see Week 5).

Worked four-P summary, the Okonkwo family.

P	Content
Predisposing	Mother the eldest of five, raised siblings from 19 after her father's death; father posted away for work since 2022; both parents raised in households where children were disciplined by senior relatives rather than by parents
Precipitating	Paternal grandmother, Ebuka's daytime carer from 2017 to 2022, died in 2023; new baby born 2024; school's first behavioural letter in the same term
Perpetuating	Ebuka's defiance escalates in the mornings; mother responds with threats she does not carry out; father, home at weekends, over-corrects; school telephones the mother, who feels judged and does not tell the father; the parents' only conversations are about Ebuka
Protective	Mother is resourceful and organised; father is affectionate on return and is not distant by choice; a maternal aunt is a trusted ally; the school's head of year is willing to meet
Help-seeking	First contact with services; the mother telephoned after the school's second letter, without telling her husband

What the table does. It shows, in five rows, why a plan of "parenting classes for the mother" would fail (the perpetuating pattern is triadic and institutional) and why a plan involving the father, the school and the bereavement is more likely to work.

9.4 A one-page relational formulation template

Services use many templates. The one below is deliberately short, because a formulation longer than a page is usually two formulations with the contradiction hidden.

1. The presenting pattern (three sentences). Who, what, when, and what has been tried.

2. The central loop (a diagram or four sentences). The cycle that maintains the problem, with the members' behaviour at each step. This should be drawable.

3. The meanings that hold the loop in place. What each member believes about the others, and about themselves, that makes the behaviour seem necessary.

4. The context. Developmental stage, health, work, money, housing, migration, culture, faith, and the institutions involved.

5. History in one paragraph. How the family arrived here, with the dates that matter.

6. What has been tried, and what it did. Including previous therapy and its ending.

7. Strengths and resources. Specific, evidenced, and usable in the plan.

8. Risk. Stated plainly, with a management plan, or stated as "no current indicators" after a screen.

9. The plan, derived. Each element of the plan should name the element of the formulation it addresses.

10. The testability clause. Two or three sentences: what would show this formulation to be wrong, when it will be reviewed, and what would be done differently if it is.

The testability clause, worked. "If the central loop is the parents' disagreement about discipline, then two weeks of a single agreed rule should reduce the morning conflict, even before any work on the child. If the mornings do not change, the loop is elsewhere most likely in the mother's bereavement, which we have not yet addressed and I will shift the focus at the review in three weeks."

9.5 Worked formulation: the Adeyemi family, six sessions in

A full formulation, written as it would appear in a clinical record after six sessions. Read it for how the sections connect, not for the content.

Presenting pattern. Amara, 16, has not attended school for four months. Mornings follow a five-link sequence (wake, refuse, insist, cry, telephone the school with an untrue explanation). The mother has contacted the school four times and has told nobody in the extended family. The father is away Monday to Friday and has attended two of the six sessions.

Central loop. Amara refuses; the mother insists and then telephones the school; the school accepts the explanation; Amara stays home; the mother spends the day angry and guilty; the father returns at the weekend and is told only the headline; the mother feels alone with it; the next morning the sequence repeats with less hope. A second loop runs

between the parents: the father's advice at weekends is experienced as criticism, so the mother tells him less, so he knows less, so his advice is less useful.

Meanings. The mother believes that a good mother manages her children alone, and that asking for help is evidence of failure; her own mother managed four children after a death, and this is the standard she measures herself against. Amara believes that her mother cannot bear her distress, and that staying home protects them both from a conversation neither can have. The father believes he has already lost standing in the household and that his contribution will be rejected.

Context. Two incomes, one of them insecure; the father's work is in another state and is not negotiable in the short term; the maternal grandmother is 71 and lives nearby; the family attends a Pentecostal church where the mother is a respected member, which raises the cost of disclosing the difficulty; Amara is in the final two years of secondary school, with examinations in eight months.

History. The mother was 19 when her father died and she raised four siblings. She married at 24 and had Amara at

1. The father's mother died when he was 16, the year he left for Lagos. The redundancy in 2019 began two years of

his being at home; the new post in 2021 restored the income and removed him from the household in the week. The school refusal began nine months after the father's return to work and three months after Amara's closest friend left the school.

What has been tried. Insistence, withdrawal of the telephone, a conversation with the school's counsellor, prayer and a fast, and a three-week period in which the mother took leave from work to supervise the mornings. All were reasonable; each was tried alone; none addressed the loop.

Strengths. The parents agree on the goal and disagree only on method; the mother is resourceful and has kept the household functioning through two difficult years; the father is affectionate and wants to be involved; the grandmother is available and willing; the school's head of year has offered flexibility; Amara has a part-time job she values and wishes to keep.

Risk. No current indicators of self-harm or suicidal ideation; no domestic violence disclosed and no indicators on screen; safeguarding referral not indicated at this stage; the school's threat to report the absences is live and is itself a stressor. Reviewed each session.

Plan, derived from the formulation. (1) Address the institutional loop first: a letter to the school written with the mother, ending the deception, and a meeting at which the therapist is present. This addresses the "meanings" section the mother's fear of being seen as a bad mother. (2) Both parents in the room for sessions 4, 5 and 7, to interrupt the weekend weekday information loop. (3) A conversation about the maternal standard of self-sufficiency, framed as history rather than as criticism. (4) A small, staged return to school, negotiated with Amara and the school, beginning with two mornings a week in a

supported room. (5) Bereavement and loss work with the mother only if she raises it, and not before the school situation is stable. (6) Review at session nine.

Testability clause. If the institutional loop is central, then ending the deception should reduce the mother's guilt and the morning escalation within three weeks, before any change in attendance. If the morning sequence does not shift, the central loop is more likely to be the mother daughter dyad, or the marriage, and the plan will be reordered at the review.

What this formulation does that an assessment would not. It commits to a lever, a sequence of moves, and a condition under which the therapist would be wrong. It could be read by a colleague, disagreed with, and improved which is the professional standard.

9.5a The network map: one case, drawn

The technique of mapping the professional network is best shown rather than described. Here is the network for the Okonkwo family, drawn in the fifth session and used in the case conference.

The nodes. Mother; father (weekends); Ebuka (9); the new baby; the maternal aunt; the deceased paternal grandmother; the school (head of year, class teacher, SENCO); the general practitioner; the clinic (therapist); the church (a pastor the mother trusts but has not told).

The links, as drawn. A thick line between mother and school, labelled "telephone, escalating"; a dashed line between father and school, labelled "nothing"; a double line between mother and aunt, labelled "support"; a conflicted line between mother and father, labelled "reports on Saturday"; a dotted line from the therapist to the school, labelled "consent given, letter sent"; and a symbol for the grandmother with a cross, positioned close to Ebuka and to the mother.

What the drawing showed that the file did not. Three things: the school had more contact with this family than any professional, and all of it was aversive; the father had no relationship with the institution that was defining his son; and the one person with moral authority in the mother's life the pastor was entirely outside the network, which meant the family's principal source of meaning and support was uninformed about the difficulty and therefore unable to help.

What changed as a result. The therapist wrote to the school with consent, naming a single contact and asking for fortnightly review by telephone with both parents; the school agreed, and the head of year spoke to the father for the first time. The father's inclusion in the institutional loop was, on the family's own account at the review, the change that mattered most. The pastor was not contacted, at the mother's request, and that decision was recorded.

The habit to build. On one page, name every institution and every significant person, draw the links, label each link with its quality and its frequency, and then ask: which link is doing the most harm, and which missing link would do the most good? The answer to the second question is usually the first thing the plan should address.

9.6 Four lenses on one family

The same case, formulated four ways. The purpose of this section is not relativism "any formulation will do" but discipline: each lens must be stated in a form that could be tested, and the therapist must be able to say why they chose this lens and what would make them change it.

The case. A couple, married eleven years, two children (7 and 4). They present with "constant arguing, no affection, and we do not know whether we should stay together". Both work; the wife earns more; the husband's mother lives with them; the wife says the husband is "never on my side"; the husband says the wife "has no respect for my mother".

Lens 1 structural. The household contains a three-generational hierarchy in which the grandmother holds executive authority over the children and the parenting, and the couple has no boundary around itself. The wife is in a coalition with the children against the grandmother; the husband is in a coalition with his mother. The maintenance mechanism is the cross-generational alliance and the absence of a clear couple subsystem.

Plan: strengthen the couple boundary (a weekly conversation with a defined agenda; decisions made by the parents before consulting the grandmother); restructure the hierarchy with the grandmother's authority acknowledged but limited to specific domains; then work on the marriage.

Test: if the formulation is right, agreeing even one domain in which the parents decide without the grandmother should reduce the arguing within three weeks.

Lens 2 emotion-focused. The arguing is a protest against disconnection. Underneath the conflict are primary emotions the wife's fear of not mattering, the husband's shame at being judged inadequate which are expressed as secondary anger. The cycle is: she criticises to be heard; he defends or withdraws to avoid shame; she escalates; he withdraws further. The arguments are the couple's way of staying engaged.

Plan: track the cycle; access the primary emotions; enact a new cycle in which she can name her fear and he can stay present; consolidate.

Test: if the formulation is right, the wife's ability to express the fear without criticism should produce a softening in the husband within one or two sessions. If he withdraws further, the formulation needs revision.

Lens 3 narrative/social-constructionist. The couple are living inside a story about what a marriage and a family should be the wife's story of partnership and equality, the husband's story of filial duty and respect, the grandmother's story of a household she holds together. The problem is not the people but the collision of stories, each of which is legitimate and neither of which has room for the other.

Plan: externalise the conflict ("the Respect argument", "the Loyalty problem"); map the influence of each story; find unique outcomes where the couple have resisted the conflict; re-author a story that can hold both duties.

Test: if the formulation is right, externalising should reduce defensiveness and allow the couple to describe their positions without accusing each other, within two sessions.

Lens 4 behavioural/cognitive-behavioural with a family frame. The couple's interaction is maintained by negative reciprocity and by cognitive attributions ("she does not respect me", "he always takes his mother's side") that are confirmed by selective attention. The grandmother's presence increases the frequency of the triggering events. There is also a skills deficit in negotiation and repair.

Plan: behavioural exchange (daily check-in, agreed small positives); attribution retraining; communication and problem-solving training; a structured conversation with the grandmother about domains.

Test: if the formulation is right, a two-week behavioural exchange should raise the rate of positive exchanges measurably and reduce the frequency of arguments.

What the comparison teaches. The four lenses identify overlapping and partly incompatible mechanisms, and each generates a plan that would probably help some couples. Three disciplines follow.

First, choose deliberately and say why. "I am working structurally first because the household hierarchy is the most immediate source of the wife's experience of being unsupported, and because a behavioural programme will not survive the grandmother's daily countermanding."

Second, write the test. Each lens above comes with a condition under which it would be falsified. A therapist who cannot state such a condition is practising a faith rather than a method.

Third, change lenses when the test fails. The most common professional failure is not choosing the wrong lens; it is keeping it after the evidence has gone the other way, because the therapist is more attached to the model than to the family.

> **DEBATE**
> If all four formulations can be defended, in what sense is formulation a professional activity rather than an artistic one? Argue that the discipline lies in the testing, not in the content.

9.7 Annotated transcript: presenting a formulation to a family

A formulation is only useful when the family can hear it. Below is the presentation of the Okonkwo formulation, six turns in, with the therapist's choices annotated.

> **T1 Therapist:** I want to tell you what I think is happening, and then I want you to tell me where I have got it wrong. Is that all right?
> **T2 Mother:** Yes.
> **T3 Therapist:** What I see is this. Ebuka's difficult mornings started in the term after his grandmother died. She had looked after him every day for five years, and nobody has really talked about that in this room.
> **T4 Mother:** (quietly) No.
> **T5 Therapist:** And I think the mornings became the place where the sadness goes. He is difficult, you are tired, the school telephones, and there is no room left for missing her. Does that fit?
> **T6 Mother:** (crying) It fits.
> **T7 Therapist:** There is a second part, and this is about the two of you. When Ebuka is difficult, you handle it alone all week. On Saturday his father comes home and hears about it, and by then it has become a report.
> **T8 Father:** She does not tell me until it is a problem.
> **T9 Therapist:** And when she tells you, what happens?
> **T10 Father:** I say what she should have done. She says I am not there. Then we are arguing.
> **T11 Therapist:** So the boy's difficult mornings have become the way the two of you talk about the week. I am not saying anyone intends that. I am saying that this is how it works at the moment.
> **T12 Mother:** So what do we do?

Annotations. T1 asks permission and sets the correction clause before delivering anything. This is not politeness; it changes the status of what follows from a verdict to a proposal, and it is the single most reliable way to avoid the family experiencing formulation as judgement.

T3 begins with a date and a person, not a concept. It uses no clinical vocabulary. The phrase "nobody has really talked about that in this room" attributes the silence to the room rather than to any individual, which is both true and safe.

T5 is the formulation itself, in three clauses, ending with an invitation: "Does that fit?" The invitation is genuine. T6 is the moment the formulation is accepted, and note that it is accepted because it is about a loss and not because it is about behaviour.

T7 T11 present the second, harder part. The therapist uses "I am not saying anyone intends that" to pre-empt the shame that would follow an account of the parents' loop, and she uses the plainest possible description ("the way the two of you talk about the week"). The formulation is delivered not as an explanation of the family but as a description of a mechanism, which is what makes it acceptable.

T12 is the question the therapist wanted. The family asking "what do we do?" is the sign that the formulation has landed: it has become useful rather than merely true.

What the therapist did not say. She did not say "Ebuka is depressed" or "this is unresolved grief"; she did not use the words "enmeshed", "triangulated" or "parentified"; she did not present the formulation as complete; and she did not offer the plan in the same

breath as the formulation, because a family that is absorbing an explanation cannot also absorb a task. The plan came eight minutes later, after the father had said something about his own mother's death that he had not said before.

9.8 Laboratory: three formulations, one transcript

The week's assessed practical. Students work from a single thirty-minute video or transcript (the case bank provides three), in groups of three, with a strict division of labour.

Stage 1 (20 minutes) three lenses. Each student writes a one-page formulation using a different lens (choose from structural, emotion-focused, narrative, behavioural, psychodynamic-systemic). All three must use the ten-part template in 9.4 and must include the testability clause.

Stage 2 (20 minutes) the challenge. Each student presents for five minutes; the other two attack the formulation on three grounds only: is the maintenance account specific? does the plan follow from it? is the testability clause falsifiable? Nothing else may be criticised.

Stage 3 (20 minutes) synthesis. The group produces one formulation that would satisfy all three critics, which usually means writing a formulation that names a primary mechanism, acknowledges a secondary one, and states what would change the therapist's mind.

Stage 4 (10 minutes) the family version. Translate the synthesised formulation into 150 words that could be said aloud to the family, with no jargon and no blame.

Marking rubric.

Criterion	Weight	Full marks
Maintenance account	25%	A specific loop with three or more elements, and one attempted solution identified
Fit to evidence	20%	Every claim traceable to something observed or reported; speculation marked as such
Plan derived	20%	Each element of the plan names the formulation element it addresses
Testability	20%	A falsifiable clause with a review point and a stated alternative
Family version	15%	150 words, plain language, no jargon, correctable by the family

The commonest failure. A formulation that reads well and cannot be tested. The marker's question is always: "what would you expect to see in the next three weeks if this is right, and what if it is wrong?" Students who cannot answer have written an essay.

9.9 Six errors in formulation

1. **The elegant formulation.** A formulation so satisfying that the therapist stops collecting evidence. Fix: write the testability clause before the formulation, not after.

2. **The single-cause formulation.** "Everything is about his mother." Families are over-determined; a single cause is usually a single lens mistaken for the whole. Fix: name a primary mechanism and at least one secondary one.
3. **The formulation by adjective.** "The family is enmeshed and the father is peripheral." Adjectives cannot be tested or changed. Fix: convert every adjective into a sequence with at least two people in it.
4. **The plan without a mechanism.** A list of interventions genogram, communication skills, a referral for the child with no account of why these and not others. Fix: delete any element of the plan you cannot connect to the formulation.
5. **The formulation withheld from the family.** A rich internal document the family never hears, so the family is working with a different story from the therapist's. Fix: present it in plain language and invite correction.
6. **The formula that never changes.** A formulation written at assessment and unchanged at discharge. Fix: date it, review it, and record what changed and why.

9.10 Culture, faith and power in the formulation

Formulation is where a therapist's assumptions about families become invisible and consequential. Four disciplines.

Name the cultural frame you are using. A formulation that treats a three-generational household as "enmeshment", or a father's involvement of his own father in decisions as "diffuse boundaries", has imported a normative model of the nuclear family and applied it as a standard. The alternative is to ask what the family's own model is, and to formulate the difficulty relative to their model: "In this family, the eldest son's duty is to consult his father before a decision of this size. The difficulty is not the consultation; it is that his wife learned about the decision last."

Take the spiritual account seriously in the formulation. Where a family attributes the difficulty to spiritual causes, the formulation should record the attribution and its consequences who in the family believes it, what it licenses, what treatment it produces, and what the family would accept as evidence of change. This is not an endorsement; it is accurate formulation. The one exception, as ever, is where the belief is endangering a child.

Formulate power and material conditions, not only relationships. A formulation that names "the couple's communication" as central when the family's actual difficulty is a husband's unemployment, an unpaid rent, or an immigration status that forbids work, has located the problem in the wrong domain and has implicitly blamed the family for a structural condition. Ask directly: "What in your circumstances, rather than in your relationships, would have to change for this to get better?"

Watch your own position. The therapist is part of the system: a professional with authority, a cultural position, a class position, sometimes a gender and generational position that mirrors or inverts the family's. The formulation should include a line about the therapist's influence where it is relevant: "I noticed that I aligned with the mother early, and that the father's silence may be a response to that alignment."

AUDIT

Take your last written formulation and count the number of claims about the family that came from an institution rather than from the family: the school's letter, the referring doctor's note, a previous report. If more than a third of the formulation's claims are institutional, your formulation is about the family's file, not the family.

9.10a Formulating across language, and with an interpreter

A formulation built through an interpreter is a formulation at one remove, and the discipline required is greater than most training acknowledges. Five rules.

Brief the interpreter on the formulation, not only on the case. An interpreter who knows that the therapist is testing a hypothesis about a mother's standard of self-sufficiency will render the relevant words with the relevant weight. An interpreter who has been told only "the family is Nigerian" will render the words literally and lose the meaning.

Ask for the family's words, and then ask what they mean. Interpreters routinely smooth idioms into clinical English: a mother's "my heart is not settled" becomes "she reports anxiety", and the therapist loses the phrase that the family could have recognised. Ask the interpreter to give you the image, then the sense, then the clinical gloss.

Never formulate from a summary. If the interpreter summarises three minutes of a grandmother's speech into one sentence, you have the interpreter's formulation, not the family's account. Ask for the long version at least once per session, and note whose sentence you are using when the formulation later goes into the record.

Check the register, not only the content. Whether a father is addressed formally or familiarly, whether a mother is spoken to as a wife or as a daughter-in-law, whether a child is included in the conversation or discussed over these are the relational facts the formulation needs, and they are usually lost in translation unless the interpreter is asked about them.

Consider who is interpreting. A child interpreting for a parent, a neighbour from the same community, or a member of the family's church all change what can be said. The professional standard is an independent interpreter, and where that is impossible, the limitation should be recorded in the formulation as a limitation of the evidence.

A final note on the therapist's own language. If you are working in a second language or a second dialect, your formulation will be built from fewer, cleaner, more formal exchanges than you would have in your first. Expect to under-hear humour, irony, proverbs and insult, and compensate by asking more directly: "When she said that, was she angry, or was she joking?" Families will tell you, and they will usually be pleased to be asked.

9.11 Evidence note: how strong is the evidence for formulation?

An honest evidence note, because formulation is the most widely taught and least empirically examined activity in the profession.

What is not established. There is no body of evidence showing that a formulation is accurate that the mechanism a therapist names is the mechanism operating. Studies of clinical judgement consistently show modest reliability and systematic bias: clinicians weight vivid and recent information, they form early hypotheses and then seek confirming evidence, and they agree poorly with each other about the same case. Formulation, being a judgement task, inherits all of these limits.

What is weakly established. There is some evidence that formulations add predictive value beyond diagnosis in some settings, and that formulation-driven treatment selection outperforms unstructured selection in routine care. The evidence is thin and heterogeneous, and it does not support strong claims.

What is better established. Two adjacent findings give formulation its rational basis. First, routine outcome monitoring with feedback improves outcomes, particularly for clients who are not progressing; a formulation with a testability clause is a low-technology form of the same discipline. Second, collaborative goal setting and agreement on tasks predicts outcome; the family-level version of this is the negotiated formulation that the family recognises as its own.

What clinicians should therefore claim. That a formulation is a provisional, testable account which organises treatment, supports shared decision-making, and is revised in the light of feedback. Not that it is a diagnosis of the family's mechanism, and not that the model which generated it is thereby validated.

The professional implication. Write formulations with markers of uncertainty, review dates and alternative explanations, and be able to say in supervision: "I am working on the hypothesis that X maintains this; if the next three weeks do not confirm it, I will move to Y." That sentence is defensible in a court, a case conference or a journal club. "The family is enmeshed" is not.

9.12 The multidisciplinary network: formulating the system around the family

Families do not live in clinics. They live in a network of institutions school, workplace, general practice, church or mosque, social services, housing, immigration, sometimes the courts and that network is part of the maintaining system. A formulation that stops at the household boundary will routinely miss the loop that matters most.

Who is in the network. Map it explicitly, in writing, for every family where more than one agency is involved.

Node	What it contributes	What it can do that you cannot
School	Daily observation; attendance data; the child's peer world	Change the classroom, the timetable, the sanctions, the narrative staff hold
General practice	Medical history; medication; the family's route into services	Investigate physical causes; prescribe; write the letter that unlocks funding
Social services	Legal mandate; safeguarding authority; resources	Remove a child, or support the family to keep one; convene a conference
Religious community	Moral authority; practical support; the family's meaning system	Legitimise the family's engagement with therapy; provide respite

Node	What it contributes	What it can do that you cannot
Employer	Shift patterns; income; the father's absence	Change a rota; grant leave
Extended family	Care, money, arbitration	Succeed where a professional's authority fails

The three network loops that recur.

The referral loop. The school refers because the child's behaviour is unmanageable; the clinic assesses the child; the report goes back to the school; the school changes nothing because the report is addressed to the child's deficits; the behaviour continues; the school refers again. The formulation must include the school's response as part of the maintenance pattern, and the plan must include something the school can do differently.

The escalation loop. A mother does not tell the father about a school letter; the school escalates to a formal warning; the mother conceals the warning; the father discovers it in a letter and experiences the concealment as betrayal; the parents' conflict increases; the child's behaviour worsens; the school escalates again. The therapist who treats only the marriage will be beaten by the institution's timetable.

The mandate loop. A family attends because a court or a child protection plan requires it; the family's participation is compliant but not invested; the therapist interprets the lack of investment as resistance; the therapist becomes more directive; the family becomes more compliant and less honest; the report says the family engaged. The formulation must record the mandate as a feature of the therapeutic system, and the therapy must be designed for a mandated family (see Week 12).

Coordinating well: six practices.

1. **Get consent in writing, and be specific about who will be told what.** "I will tell the school about the plan and about your son's strengths. I will not tell them about your marriage."
2. **Hold one case conference rather than six telephone calls.** A single meeting with an agenda, a decision log and named actions is worth more than a month of coordination.
3. **Send the family the minutes, or read them aloud.** Families are routinely discussed by professionals about whom they know nothing, and this is both an ethical and a clinical error.
4. **Agree who holds the risk, and record it.** Confusion about mandate is the single most common cause of serious case failures.
5. **Do not confuse coordination with agreement.** Professionals can disagree productively if the disagreement is recorded and the family is not caught in the middle.
6. **Keep the family's own network in view.** The aunt, the pastor, the coach and the grandmother are often more influential than any agency, and they cost nothing to involve; a plan that ignores them will be quietly undone at home, usually within a fortnight and usually kindly, by people who were never asked and who would have helped gladly.

CASE

A 13-year-old is referred by his school for "aggressive behaviour". The clinic sees the family, finds a recent bereavement, and offers six sessions. Meanwhile the school has issued two exclusions, the mother has changed her telephone number to avoid the school's calls, and the father has been told nothing. The formulation that matters is not "unresolved grief"; it is "a school and a family escalating each other while a boy grieves", and the plan begins with a meeting at the school, not with a session in the clinic.

9.13 Writing it: the record, the report and the letter

Formulations circulate. They go into the clinical record, into reports for courts and schools, into referral letters and, sometimes, into the hands of the family. Three formats, three disciplines.

The clinical record. Write for three readers: your future self, a colleague covering your leave, and the family if they request their records. That means plain sentences, no abbreviations that only your team understands, and no speculation presented as fact. Use the phrase "my hypothesis" where it is one, and "the family reports" where that is the source. Date every formulation and note the review date.

The report for an external body. Courts, schools and local authorities want three things: what you were asked, what you found, and what you recommend. Put the formulation in one numbered paragraph in the findings, keep the theory out of the recommendations, and make each recommendation concrete, achievable and attributable to a named person. Avoid adjectives about the family ("chaotic", "enmeshed", "manipulative") they will be quoted back at the family in a meeting you do not attend.

The letter to the family. Increasingly standard, and a useful discipline. A one-page letter after the assessment containing four things: what you understand the difficulty to be, in their words; what you think is keeping it going; what you propose; and what you would want to know if you were them. Write it as you would write to a colleague you respect. If you cannot, you have not understood the case well enough.

A worked paragraph, for a school. "I met with Ebuka and his mother three times between March and May. Ebuka is a nine-year-old who lost his grandmother in 2023; she had cared for him daily for five years. His difficulty in the mornings began in the term after her death. In my assessment the mornings are the place where the loss is expressed, and the school's letters, while understandable, have increased the pressure on his mother at exactly the point where she needs support. I recommend: (1) that a named member of staff meet Ebuka weekly for six weeks; (2) that attendance be reviewed fortnightly with his mother rather than by letter; (3) that I continue to see the family and report to the school with their consent in June."

What that paragraph does. It gives the school a formulation, a rationale, three actions, and a date. It does not use a single clinical term. It treats the school as a partner with a legitimate concern, and it locates the school's own behaviour in the pattern without blaming it.

9.14 Key terms

Term	Definition
Assessment	The systematic gathering of information about a family and its difficulty
Formulation	A provisional, testable explanation of the difficulty and what maintains it
Predisposing factor	A feature of history that makes the difficulty more likely
Precipitating factor	The event, and its meaning, that precedes the onset
Perpetuating factor	What maintains the difficulty now
Protective factor	A resource that reduces risk or supports change
Maintenance loop	The cycle of behaviour and meaning that keeps the problem in place
Testability clause	A statement of what would show the formulation to be wrong
Differential formulation	An alternative account that the evidence does not yet exclude
Network	The set of institutions and people involved with the family
Mandate	The authority under which the work is done, and by whom
Case conference	A multi-agency meeting that coordinates a plan

9.15 Self-test

1. What are the five jobs of a formulation?
2. Why must a formulation include a maintenance account?
3. Rewrite "predisposing factor" in relational terms.
4. What is a testability clause, and why does it matter?
5. Name two of the three network loops described in this week and explain how each can defeat a good formulation.
6. What are three errors in writing a formulation for an external body?
7. What does the evidence say about the accuracy of formulation?
8. What must a formulation contain where a family holds a spiritual explanation?

Answers. 1. Organise information; explain maintenance; predict; generate a plan; be revisable. 2. Because explaining onset does not tell you what to change now, and the maintenance pattern is usually the lever. 3. The family's relational history that makes this kind of difficulty more likely patterns of managing stress, loss, authority and intimacy transmitted across generations. 4. A statement of what would show the formulation to be wrong, when it will be reviewed, and what would be done differently; it converts a belief into a hypothesis. 5. The referral loop (the institution's response becomes part of the problem and is omitted from the formulation) and the escalation loop (institutional deadlines outpace the therapy); the mandate loop is the third. 6. Adjectives about the family, theory inside the recommendations, and recommendations without named owners or dates. 7. Accuracy is not established; clinical judgement has modest reliability and known biases; the better-supported claim is that testable, monitored formulations support

treatment selection and feedback. 8. The attribution, who holds it, what it licenses, what the family would accept as evidence of change, and the limits imposed by safeguarding.

9.16 Reflective task

Write a one-page formulation of a family you have observed from placement, a recorded session, or the case bank and then write the testability clause. Now write a second paragraph answering: if I am wrong, what are the two most likely alternatives? Bring all three to the seminar. The exercise is uncomfortable for most students, because the professional training rewards confident explanation and this task rewards acknowledged uncertainty. That discomfort is the point, and it is worth naming in the seminar: the capacity to hold a hypothesis firmly enough to act on it and loosely enough to drop it is the whole of clinical judgement, and it is learned by practising the discomfort rather than by resolving it.

9.17 Readings for this week

- **Carr (2006)**, Family Therapy: Concepts, Process and Practice, the chapter on assessment and formulation, for the standard structures and their variants.
- **Johnstone & Dallos (2014)**, Formulation in Psychology and Psychotherapy, for the critical and comparative treatment; the chapters on systemic formulation are directly relevant.
- **Dallos & Vetere (2009)**, Systemic Therapy and Attachment Narratives, for an integrative approach to formulation across models.
- **Stratton et al. (2015)**, on the SCORE-15 and on idiographic measures how to bring the family's own evaluation into the formulation.
- **A safeguarding review or serious case review** published in your jurisdiction, read for how formulations fail at the boundary between agencies.
- **A published court report or expert witness statement** in a family case, read for how formulation is written when it will be tested in public. Note how little theory appears, how much evidence, and how carefully every recommendation is tied to a named person and a date.
- **One serious case review and one successful family intervention report**, read side by side, for the difference between a network that shared information and one that shared only forms.

Next: Week 10 Culture, class and power

Week 10 Culture, class and power

Every family you meet is embedded in a culture, and so are you. Culture is not a variable to be "controlled for"; it is the medium in which the problem is defined. The same is true of class and of power, and the three are entangled: a family's account of a difficulty, the institution's account, and your account of both are all shaped by where each party stands.

10.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0 10	Three mistakes	Culturalism, colour-blindness, essentialism named and illustrated	Identify each in a real referral
10 25	The cultural genogram	Draw one live, with the class's own material anonymised	Score their own draft genogram
25 40	Class and money in the room	Two cases, same symptom, different class position	Predict what each family will be offered
40 55	Power in the session	Who speaks, who is translated, who is interrupted	Count interruptions in a recorded session
55 70	Faith as the organising frame	Working with a pastor and an imam	Plan a coordination meeting
70 85	The limits of cultural respect	A case where culture is used to defend harm	Write the safety plan and the referral
85 90	Set the assignment	The cultural genogram and commentary	Plan the interview

10.2 Three mistakes

Most failures in cross-cultural clinical work are not failures of knowledge. They are failures of method, and they cluster into three recognisable errors.

Mistake 1 culturalism. Treating culture as a set of rules that predict individual families: "Nigerian families are hierarchical", "Asian families do not discuss emotion", "in this culture the grandmother decides". Culturalism is attractive because it promises competence without inquiry, and it is harmful because it turns a hypothesis into a fact, relieves the therapist of asking, and makes the family a specimen of a category. It also flattens enormous within-group variation: two Ibadan families can differ more from each other than either differs from a family in Leeds.

Mistake 2 colour-blindness. The opposite error: treating everybody as the same, refusing to notice that culture, ethnicity, class and religion shape both the difficulty and the encounter. The therapist who says "I do not see race" has usually failed to notice that the family does, and that the family's experience of institutions schools, hospitals, police, social services has taught them something about how they will be seen. Pretending not to see difference is a way of asking the family to do the work of managing the therapist's discomfort.

Mistake 3 essentialism about the family's own account. Once a family has described their culture, the trap is to treat that description as fixed. Families describe their culture strategically, to the person in front of them, and for reasons: to claim authority, to protect a member, to explain a decision, to test whether the therapist will judge them. "In our culture, a wife does not contradict her husband" may be a description, an aspiration, a negotiating position, or a warning and the therapist who takes it as a law will be unable to work with the wife who contradicts her husband in the second session.

The alternative to all three. A disciplined curiosity: ask, note, check, and revise. The formulation in Week 9 is the place where this discipline lives, because a formulation records both the family's account and the therapist's hypothesis about it, and distinguishes the two.

10.3 The cultural genogram

The genogram of Week 8 maps kinship. A cultural genogram maps the content carried through that kinship: language, religion, migration, education, occupation, class, and the values each generation transmits and contests.

The seven dimensions to map.

1. **Migration history.** Who came from where, when, why, and what was lost. For a family in Nigeria this may be internal migration (rural to city, state to state) as often as international.
2. **Languages.** Which language is spoken to whom, in which room, about what. The language in which a mother scolds is often not the language in which she consoles.
3. **Religion and religious change.** Who practises, who converted, and who disapproved. A grandmother's traditional practice alongside a granddaughter's Pentecostalism is a live cultural fact in many households.
4. **Education.** Who was educated, to what level, and what education means as a family achievement.
5. **Occupation and class.** Including the family's own class story "we are not rich, but we are respectable" and the class position of previous generations.
6. **Duty and individuation.** What a child owes a parent, and at what age; what a wife owes a husband's family; what is expected at a funeral, a wedding, a naming ceremony.
7. **Stigma and disclosure.** What may not be spoken about outside the family, and to whom: mental illness, debt, a divorce, a child born outside marriage, a prison sentence.

How to draw it. Use the genogram's structure, with annotations: a language written beside each person, a symbol for religious practice, a note for migration, arrows for conversion, a dotted line for a cut-off caused by a culturally significant event. Then ask three questions: What did each generation hand on that the next generation values? What did it hand on that the next generation rejects? Where is the disagreement now?

Why this matters clinically. Most family presentations that look like "communication problems" are, on closer inspection, disagreements between generations about duty, respect and autonomy and those disagreements are cultural in the precise sense that they concern which values a family is going to live by. A genogram that maps the values makes the disagreement visible and, often, discussable for the first time.

PRACTICAL

Draw a cultural genogram of your own family across three generations, with the seven dimensions. Then write one sentence for each dimension beginning "In my family, this was never discussed". Those sentences are the material you will find hardest to ask about in a family that is not yours.

10.4 Class, money and material conditions

Therapy is a middle-class activity conducted in middle-class language, and families who are not middle-class are routinely misread by it. Five mechanisms.

The presentation is shaped by scarcity. A family under financial pressure will often present a behaviour problem and not a marital problem, because behaviour can be fixed with a rule and a marriage cannot be fixed without time, privacy and energy that the family does not have. The therapist who insists on the underlying relational issue may be right and may also be privileged.

Attendance is a class issue. A missed appointment is read as resistance; it is frequently a bus fare, a shift change, a childcare problem, or a job interview. Before interpreting a non-attendance, ask what made it hard to come.

Time is not equally available. Homework, daily diaries and "a conversation every evening" assume evenings. For a family with two jobs, shift work, or a crowded household, the intervention must fit the actual week, and the therapist should ask what the week looks like before prescribing anything.

Language registers communicate class. The therapist's vocabulary, the form, the waiting room, and the length of the sessions all tell the family where they are and who they are dealing with. Families respond accordingly, often with politeness that is not agreement.

Aspiration and shame. Many families are managing a gap between the life they were promised and the life they have, and between their own position and that of relatives who have done better. Questions about work, housing and school fees can touch shame very quickly, and a therapist who asks about money as though it were neutral will get answers that are shaped by the shame.

Three practical moves.

Ask about money directly and without embarrassment. "How are things financially at the moment?" is a clinical question, and families answer it more often than trainees expect.

Ask what the week actually looks like. Hours worked, who is at home when, who sleeps where. The intervention that fits the week will be followed; the one that assumes a different week will not.

Locate the problem in the conditions when the conditions are the problem. A father who is absent because he works six days a week in another state is not "disengaged"; he is absent, and the plan must work with the absence rather than interpret it.

10.5 Power in the room

The therapy room is not a democracy. The therapist has the authority of the institution, the language of the profession, and the power to write the record. The family has the knowledge and the choice of whether to return. Making this explicit, at least to yourself, is a precondition of ethical practice.

Five sites of power, and what to do about each.

Who defines the problem. Already discussed in Week 7, and it is a power question as well as a clinical one. The parent who telephones usually defines; the adolescent who is brought usually does not. Ask the person who has not spoken.

Who is interpreted. The member whose speech is summarised by others in the room usually the youngest, the oldest, the least fluent, or the one whose first language differs from the therapist's. Notice who is spoken about rather than to, and address them directly.

Whose emotions are permitted. In many families, anger is permitted to the father and tears to the mother, and neither is permitted to the child. The therapist who mirrors the family's permissions will reproduce them. Ask the silent member what they would say if they were allowed to be angry.

Whose account is believed. Where two members give incompatible accounts, the therapist's attention will drift towards the more articulate, the more distressed or the more culturally familiar. Record both accounts, and say aloud that there are two.

Who is the expert. The professional's authority is real and should be used, not denied. The question is whether it is used to deliver a verdict or to structure a process in which the family's knowledge can operate. The difference is visible in the sentence "I think we should try X" versus "what would make X possible in your house?"

> **AUDIT**
> Record fifteen minutes of a role-played session. Count: how many questions were addressed to each member, how many times each member was interrupted, and whose words the therapist repeated. The pattern will not be random, and it will tell you more about your own cultural positioning than any reading.

Four practical disciplines.

1. **Ask rather than assume.** "In your family, who usually makes the big decisions? Who is consulted? Is that how you want it to be?"
2. **Locate yourself.** Complete the same cultural genogram for your own family. Your views on hierarchy, individuation, duty and disclosure are not neutral.
3. **Notice the power in the room** including yours. The family may be polite because of your status, not because they agree.
4. **Interrogate "normal".** Models that prize individuation can pathologise families who prize interdependence. Ask what the family's own picture of a good life is.

The four are the minimum standard for this week, and they are easy to agree with and hard to practise: each requires the therapist to be uncertain in public, and professional training rewards the opposite.

10.5a Structural competency: when the problem is the institution

"Culturally sensitive" practice has a blind spot: it locates difference in the family and neglects the institutions that are shaping the family's problem. Structural competency is the discipline of noticing the second.

What it means. The capacity to recognise how housing, employment, immigration status, school admissions, policing, benefit rules and waiting lists produce the presentations that arrive in the clinic and to act on that recognition rather than translating it into psychology.

Three questions that reveal structure in an ordinary session.

> "What would have to change outside this family for this to get better?" "Which of the things you have told me are decided by somebody else?" "When you have asked for help before, what happened?"

The answers are frequently decisive. A family whose evenings are unmanageable because four people share two rooms is not a family with a boundary problem. A father who is absent because his leave is refused is not disengaged. A mother who misses appointments because the clinic bus runs twice a day is not resistant.

The clinical consequence. Where the problem is structural, the intervention should include an institutional move: a letter, a telephone call, an advocate, a benefits check, a referral to a housing service, a school meeting. This is not social work instead of therapy; it is therapy that has formulated accurately. And it is often what the family experiences as most respectful, because it treats their account as true.

The limits. The therapist cannot fix a housing system, and promising to do so is a way of failing the family twice. The professional move is to be precise about what can be changed, honest about what cannot, and explicit in the formulation: "We cannot change the accommodation in the next three months. We can change how the evenings are organised within it, and I will write to the housing office with your consent."

10.6 Worked case: when duty and desire collide

A case that requires culture, class and power to be formulated together.

The referral. A 21-year-old woman, Tolu, brought by her parents to a family clinic. The parents' complaint: "she has refused the man we chose for her, and she wants to marry a boy from another tribe whom she met at university." Tolu's complaint, offered when asked directly: "they are selling me." The parents are Yoruba, from a modest household in Ibadan; the father is a retired teacher, the mother a trader. Tolu is the eldest of four and the first to attend university; the family has paid her fees with contributions from two uncles. The proposed husband is Ibo, a fellow student whose family the parents have never met.

What a culturalist reading would produce. "Arranged marriage is normative; the daughter's refusal is an acculturation conflict; she should be helped to respect her parents' values." This reading is respectful in tone and useless in practice: it takes the parents' description of the custom as the whole truth, ignores the daughter, and assumes a homogeneity that the family itself does not have.

What a power-blind reading would produce. "Forced marriage is a human rights violation; the daughter must be supported to leave." This reading is ethically serious and clinically naive: it treats a contested family process as a crime already committed, forecloses the parents' real authority and affection, and hands the family a choice between therapy and their standing.

The formulation that makes work possible. Tolu's refusal is not a rejection of the family but a disagreement about a decision the parents experience as theirs by right. The parents' position is not coercion in their own understanding: it is duty, it is protection (they believe a chosen husband from a known family is safer than a stranger), and it is standing (a marriage into a family they do not know is a public statement about their competence as parents). The uncles' contribution to Tolu's fees is a live obligation that nobody has named. The mother has reservations that she has not said in front of the father. Tolu has not explained what she wants, because she expects to be told she is ungrateful.

What the therapist does, in order.

First, hears all three accounts separately and without adjudicating. The parents, the daughter, and critically the mother alone. In the individual conversation the mother says that she was herself married to a man she had met twice, and that she has never said this to her daughters.

Second, names the difference between duty and coercion, in language that does not accuse. "Your parents are telling me what they owe you and what you owe them. You are telling me what you want. At the moment nobody in this room is talking about the same thing. Let us start with what each of you is protecting."

Third, refuses the false choice. The therapist says directly that she will not help to force a marriage and will not help a daughter to leave her family. Both statements are made to everyone. This is the ethical floor, and it is also strategically effective, because it makes the parents' continued attendance a choice rather than a defeat.

Fourth, finds the negotiable territory. Three things emerge: the parents' real concern is that the suitor's family is unknown, not that he is from another tribe; the uncles' obligation can be addressed by repayment over time; Tolu is willing to delay any marriage for a year and to have the suitor's family visit.

Fifth, sets a task that keeps everyone in the room. A meeting between the two families, facilitated by the therapist, with the father's consent and the mother's support; an agreement that no marriage will be arranged during the year; a plan for Tolu's fees.

What made it work, in system terms. The therapist took culture seriously without treating it as a law; she located the parents' position in duty and standing as well as in

custom; she brought in the uncles' obligation, which was material and unspoken; she found the mother's private dissent, which changed the family's internal balance; and she made the ethical limit explicit in a way that did not require anybody to be humiliated.

Where it could have failed. Had the parents been planning a marriage against Tolu's will within weeks, the plan would have required safeguarding and possibly legal advice, and the therapeutic timeline would have had to give way. Had Tolu's suitor been violent, the same. Culture is a legitimate frame for understanding, and it is never a reason to leave someone unsafe.

10.7 Annotated transcript: naming a cultural assumption in the room

An eight-turn excerpt from the Tolu case, session two, with both parents and Tolu present.

> **T1 Father:** In our culture a child does not choose. We chose for her because we know the family. She is bringing shame.
> **T2 Therapist:** When you say our culture, you mean the way it was done for you as well?
> **T3 Father:** Yes. I did not choose my wife. We have been married thirty-one years.
> **T4 Therapist:** Thirty-one years. And it worked.
> **T5 Father:** It worked.
> **T6 Therapist:** I want to understand what "worked" means for you, because I think that is the part your daughter has not heard.
> **T7 Father:** (pause) It means we did not divorce. It means my children grew up in one house with two parents. It means people respect us.
> **T8 Therapist:** (to Tolu) Did you know that was what he meant?

Annotations. T2 is the central move of the segment: it converts "our culture" from a general law into one man's biography, without contradicting him. The father's answer at T3 gives the therapist the material that matters thirty-one years and it is offered proudly, which means the therapist can work with it.

T4 is a simple acknowledgement, and it is doing something specific: it treats the father's marriage as a success rather than an arrangement to be critiqued. Trainees often cannot bring themselves to say this, because they disagree with the practice. Saying it costs nothing and buys the right to ask the next question.

T6 names a gap in the family's conversation rather than a gap in the father's reasoning. The phrase "the part your daughter has not heard" is addressed to the relationship, not to the father's position, and it is true: Tolu had never heard her father say what a marriage was for.

T7 is the father's own account of what he is protecting, in three clauses: continuity, respectability, and the avoidance of divorce. Any of these can be worked with. "Because it is our culture" cannot.

T8 turns to the person who has been spoken about rather than to, and it asks a question whose answer the therapist genuinely does not know. When Tolu said "no", the session changed course: she had not known, and the knowledge made her father's position human without making it right.

What the therapist avoided. She did not ask the father whether he had loved his wife; that question would have been intrusive and would have converted a cultural negotiation into a romantic one. She did not point out the inconsistency between "we respect our elders" and the uncles' money. And she did not tell Tolu that her father's position had merit in front of him, which would have made Tolu feel abandoned by the therapist; she said it to Tolu later, alone.

10.7a Working with a community or religious co-worker

Many families are best served by a therapist working alongside somebody from their own community: a pastor, an imam, a community health worker, a respected elder, a cultural broker. The arrangement is powerful and requires care.

What the co-worker brings. Trust, language, moral authority, knowledge of the family's standing, access to networks of practical support, and the ability to say things the therapist cannot say and be heard.

What the therapist brings. Confidentiality, a formulation, a method, the capacity to hold a boundary with an institution, and the professional accountability that the co-worker does not have.

Five conditions for the partnership to work.

1. **A written, shared understanding of confidentiality.** Who will know what, and what will not be repeated. This is the condition most often skipped and most often fatal.
2. **Clarity about roles in the room.** Who leads, who sits where, who speaks to whom. Families read these arrangements instantly, and a muddled arrangement undermines both authorities.
3. **Agreement on the safety floor.** The co-worker must know, before the first session, what the therapist is obliged to do if a child or a partner is unsafe, and must not contradict it in front of the family.
4. **A way to disagree privately.** The therapist and the co-worker will disagree about a marriage, a discipline, a disclosure and the disagreement must have a place to go that is not the session.
5. **A review.** Partnerships drift; a fifteen-minute review every four sessions catches the drift before the family does.

The risk to name. A co-worker who is a member of the same congregation or community cannot offer the family the confidentiality that a clinic can, and a family may prefer the clinic for exactly that reason. The therapist should not assume that community embedding is always preferable; ask the family which they want, and record the answer.

10.8 Faith as the organising frame

In much of the world, faith is not an "aspect" of family life; it is the frame in which family life is understood. Four practices for working with it well.

Ask what the faith says about the problem. Not in general, but in this family's version: is the difficulty a test, a punishment, a spiritual attack, a consequence of disobedience, or

an opportunity? The answer determines what treatment the family has already tried and what they will accept from you.

Ask who in the religious community knows, and who they want involved. A pastor or imam may be the family's most trusted authority, and may be entirely unaware of the difficulty, or may be deeply involved. Both are workable; not knowing is not.

Coordinate rather than compete. The failure mode is the therapist who implies that prayer is a substitute for change, or the religious leader who implies that therapy is a substitute for faith. The working position is complementary: "I will work on what happens in your house between Monday and Saturday; your pastor will do what he does; we will not contradict each other, and if we do, we will talk."

Attend to the cost of disclosure. A family that is prominent in a congregation may experience the difficulty as a threat to standing, and may therefore present late, minimise, or refuse a diagnosis. The formulation should record this, and the plan should protect the family's standing where doing so is compatible with safety.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent twice. She does not want the police involved and does not want her family told. The therapist must hold three things at once: respect for her autonomy and faith, a safety plan that does not depend on him changing, and honesty about the limits of confidentiality. (Full treatment in MFT 712 and 724.)

The principles, stated once. The woman's safety is not negotiable; her choices about disclosure are hers unless a child is at risk; the pastor's involvement is a resource if he is willing to be bound by the same safety principles; and the therapist's honesty about the limits of confidentiality is required before, not after, the disclosure.

10.9 Discrimination, tribe and the family's experience of institutions

Families arrive with a history of how institutions have treated people like them, and that history is part of the clinical picture.

Ask about it, briefly and directly. "Has anything like this happened before, with a school or a clinic? How was it for you?" The answers often explain a presentation that looks like hostility or withholding: a father who is curt with a professional may be managing a lifetime of being treated as a suspect.

Name what you notice, without claiming authority over it. If a family describes being treated badly because of their ethnicity, tribe, religion, accent or class, the therapist's job is to believe the account as an account, to explore its consequences, and not to adjudicate its accuracy. "That sounds as though it happened more than once" is more useful than "are you sure that was the reason?"

Watch your own reactions. Discomfort, over-compensation, curiosity about difference, and the wish to be seen as an ally are all common and all visible to families. The therapist

who works harder with a family because of their background is treating them differently, which is the problem in a different form.

Do not make the family responsible for educating you. Questions about culture are legitimate; a session spent interviewing a family about their traditions is an extraction. Use supervision and reading for general knowledge, and the session for this family.

Tribal and religious difference inside the family. In Nigeria, as elsewhere, the sharpest cultural conflicts are often internal: a marriage across ethnic lines, a conversion, a chieftaincy dispute, a family divided between denominations. The formulation should locate the conflict precisely, because "culture" as a single word conceals which culture, whose version, and against whom.

10.10 Language, naming and address

Language is the most immediate marker of power in a session, and it is where trainees make the most avoidable mistakes.

Names. Ask how each person wishes to be addressed, and use it. "Mrs Okonkwo" and "Chidi" and "Mummy" carry different relationships, and the therapist's choice will be read as a claim about the relationship. Where a family uses kin terms "Aunty", "Baba", "Mummy" for people who are not kin, follow the family's usage rather than correcting it; those terms are the map of obligation.

Address and formality. In many families the therapist's use of a first name for a father is a small presumption that will be noticed and not mentioned. Offer the choice: "I am happy to be called by my first name; what would you prefer?" The offer itself signals that the therapist knows the difference exists.

Whose language the session is conducted in. If the family thinks in their first language about emotional matters, the session may need that language or an interpreter. Interpreters are not furniture: brief them, seat them beside you rather than between you and the family, ask them to interpret speech rather than summaries, and never ask them to translate a disclosure you have not prepared for. (The discipline is set out in Week 9; the cultural point here is that language choice is itself an intervention a family asked to speak about grief in a second language may be protected from the feeling, or prevented from reaching it.)

Proverbs, idioms and images. Many families speak in proverbs, and a proverb is a compressed theory of the problem. "A house that is not swept inside will be swept by the wind" is a statement about privacy, shame and outside interference. Ask what the proverb means, and use it later in the work; a formulation that can be expressed in the family's proverb is a formulation the family will remember.

Silence and indirectness. Some families regard direct questions about feelings, marriage or money as rude, and some regard indirectness as evasion. The therapist's task is to match the family's register while still obtaining the information, which usually means asking the question in a form that protects face: "Many families find this difficult to talk about; is it like that for you?" is a direct question with a face-saving first clause.

10.11 Gender, hierarchy and the limits of cultural respect

This is the hardest section of the week, and the one on which students most often look for a rule that does not exist.

The principle. Culture is a legitimate frame for understanding a family, and it is never a justification for harm. The professional's obligations are set by law and by the code of conduct, not by the family's account of custom. Where the two conflict, the obligation wins, and the task becomes how to act on the obligation while preserving as much of the family's dignity and engagement as possible.

Where the tension appears. Physical chastisement of children; child marriage; female genital cutting; a wife's economic dependence and her lack of authority over her own movement; the discipline of a daughter by senior relatives; the treatment of a child believed to be a witch; the refusal of medical treatment in favour of prayer; the concealment of a relative's mental illness to protect the family's standing.

What to do, in order.

First, know the law and the guidance in your jurisdiction, precisely. Not approximately. The therapist who believes wrongly that a practice is "cultural and therefore permitted" is a liability, and the therapist who reports without knowing the threshold is also a liability. This is examinable knowledge, and it belongs in supervision.

Second, distinguish the practice from the family's meaning. Most parents who hit their children are not cruel; they are frightened, unsupported, and following a model that was used on them. Naming the meaning does not excuse the practice; it makes the practice changeable.

Third, be transparent before acting. Where a report or a referral is required, say so, in advance, in plain language, and say what you are going to do and what will happen next. Families forgive a professional who told them the truth and acted on it; they do not forgive being reported in secret.

Fourth, do not abandon the family after the referral. The referral is one move in the work, not the end of it. Where it is legally possible, continue to see the family, and make the continuation conditional on the practice changing.

Fifth, get support. These cases are the ones that produce moral injury in clinicians. Take them to supervision, and do not carry them alone.

> **CAUTION**
> Do not use "we must respect the culture" as a way of managing your own discomfort about acting. A therapist who leaves a child in a harmful situation because intervention felt culturally intrusive has not been culturally sensitive; they have been silent.

And the mirror image. Do not use safeguarding as a way of avoiding cultural understanding: the family that is reported within twenty minutes of the first appointment, without the therapist having asked what the practice means, how the family understands it, or what alternatives exist, has been failed twice.

10.11a Worked decision: a discipline case at the threshold

The abstract principle becomes concrete in the case that trainees most often get wrong.

The presentation. A couple attends about their 11-year-old son's behaviour at school. In the second session the mother mentions, without emphasis, that the father "flogs him when he is rude, the way we were flogged", and that the school has noticed marks on his legs. The father is calm, affectionate towards the boy, and puzzled by the referral: "In this house we correct our children. That is not abuse."

What the therapist knows. Corporal punishment is lawful in some jurisdictions and unlawful in others, and the school has a safeguarding duty that will be exercised whether or not the therapist acts. The therapist's own code of conduct requires that a child's welfare is paramount, and that concerns are reported where the threshold in local guidance is met. The father's account is congruent with his own upbringing and with the practice of most families in his community. The mother is worried but loyal, and has said more than she intended to.

The wrong moves. The cultural deferral: "This is how the family does things; I will explore the meaning." This ignores the marks, the school's duty, and the law. The immediate report without conversation: defensible in some jurisdictions where the threshold is met by marks, but undertaken without telling the family, which ends the therapy and teaches the family to conceal. The psychological translation: treating the flogging as an expression of the father's own childhood and offering insight, while the child remains at risk this week.

The sequence that is both lawful and clinically sound.

One: screen and document, factually. What happens, with what, how often, what marks, when last. In the child's presence where appropriate, and separately where not. Record what was said, not what it means.

Two: state the position plainly, to both parents, in the first conversation. "I have to tell you two things. The marks on your son's legs mean I have to act under the safeguarding rules; I am going to speak to the school and to the safeguarding lead today, and I will tell you exactly what I say. And I want to keep working with you, because what happens in your house on a Tuesday evening is something we can change together."

Three: act, and tell the family what was said. Transparency before action, and a report that is factual and non-prosecutorial in tone, is both lawful and more likely to preserve the engagement.

Four: offer the alternative now, not later. Most parents who use corporal punishment do so because they have no second method that they believe in. Give one: a specific, culturally acceptable alternative to the specific incident the removal of a privilege, a written rule agreed with the child, a cooling-off routine, the involvement of a respected relative and rehearse it in the session.

Five: remain the family's therapist where it is possible. The family that is reported and then abandoned learns that services punish and disappear. The family that is reported,

told the truth, and then helped to change learns something more useful, and so does the child.

The professional's own support. Cases at the threshold generate anxiety, and anxiety generates either over-reporting or avoidance, and both are failures of the same kind: the therapist's discomfort has replaced the child's welfare as the organising concern. Take the decision to supervision before acting where there is time, and record the advice received. The decision is the therapist's, and so is the accountability; being able to show the reasoning, the advice taken and the family's own account of what was said is the difference between a defensible decision and an unexplained one.

The generalisation. Almost every cultural conflict in clinical work has this shape: a practice the family understands as care, a professional obligation, a child or partner who is not in a position to consent, and a therapist who must act without humiliating the people they are trying to help. There is no formula, but there is a sequence, and the sequence is: know the threshold, document the facts, tell the family the truth, act, stay.

10.12 Migration and the acculturation gap

Migration produces a predictable family dynamic, and it is worth teaching explicitly because it appears in the referral as "conflict between parents and adolescent".

The gap. Parents and children acculturate at different rates. Children acquire the new culture quickly, through school and peers, and often become the family's interpreter, negotiator and public face. Parents acquire it slowly, and often deliberately preserve the values of home as an act of continuity and self-respect. The result is a family in which the adolescent knows the new rules better than the parents and is still expected to obey the old ones.

The pattern in the clinic. The young person is referred (or brought) for defiance, low mood, self-harm or school refusal. The parents describe a child who has "become different". The young person describes parents who "do not understand anything". The therapist who takes either side loses the other, and the family's actual difficulty that the rules are being negotiated in a household where the two generations have different maps remains unformulated.

What helps.

Make the gap the client. "In this house there are two sets of rules, and nobody has decided which applies to what. That is not a failure in any of you; it is what happens when a family moves." Externalising the conflict this way converts a contest about loyalty into a shared problem, which is the only version that a family can solve.

Separate the domain from the value. Parents are usually willing to negotiate domains bedtime, dress, money, going out while defending values respect, chastity, family duty, education. The therapist's task is to find the domain trade: what the young person may decide, and what remains a family matter. Negotiating values directly usually fails, because it looks like asking the parents to stop being who they are.

Address the parent's own loss. Migration involves losses that are rarely named: status, language competence, friendship networks, the extended family that would have shared the parenting. Parents who are grieving a lost world are less flexible than parents who are not, and the grief is workable when it is named.

Attend to the children left behind and the children who came later. Sibling groups that were separated by migration have their own history: the child who was raised by a grandmother and rejoined the family at eleven has a different relationship to the parents than the sibling born after migration. This is often the buried structure of the presenting problem, and the genogram of Week 8 shows it in ninety seconds.

10.12a Two migration cases, worked briefly

Case 1 the reunited family. Parents migrated when their son was two; he was raised by his grandmother and joined them at fifteen. The referral is "defiance and stealing". The formulation is not delinquency: it is a family in which the person who raised the child is absent, the parents are strangers with authority, and the child has lost his language, his school standing and the grandmother in a single move. The intervention begins with the boy's account of the grandmother, a telephone arrangement that keeps her in his life, an explicit negotiation of what these two adults are entitled to ask of him and how quickly, and a plan with the school for the first term. The stealing, it emerges, began when he had no money and did not know how to ask.

Case 2 the second generation. A 17-year-old girl, born in the city to parents who migrated before her birth, referred for self-harm after her parents discovered a relationship. The parents' account is of a daughter who has betrayed the family; the daughter's is of a family that watches her. The formulation holds both: the parents are protecting a daughter's honour in a community where the family's standing is their children's reputation, and the daughter is managing two sets of rules with no adult who can help her hold both. The intervention includes separate sessions, a negotiated agreement about what the parents need to know and what they do not, a risk plan, and where it can be found a trusted adult from the family's own community who can hold the parents' anxiety without endorsing surveillance.

What the two cases have in common. In both, the "culture" is only half the story; the other half is the practical conditions of migration housing, school, work, money, status and the family's grief for a world it has lost. A formulation that names only values will miss the conditions, and a plan that addresses only conditions will miss the values.

10.13 Evidence note: does cultural adaptation improve outcomes?

A short and useful evidence note, because "culturally adapted" is a phrase used loosely.

What the research shows. Meta-analyses of culturally adapted interventions mostly in individual therapy for adults find modest but consistent benefits of adaptation over unadapted interventions, with larger effects where the adaptation is deep (engaging the client's cultural values and world view) rather than superficial (translating materials and matching the therapist's ethnicity). The strongest evidence is for language matching and

for adaptations that address engagement and retention rather than for adaptations of specific techniques.

What is not established. That matching therapist and family ethnicity improves outcomes by itself; the evidence is mixed, and the mechanism appears to be the family's expectation of being understood rather than the match itself. Nor is it established that an unadapted, evidence-based model fails with families from other cultures; the more common finding is that it works less well, for reasons largely to do with engagement.

What this means for practice. Adapt the engagement deliberately language, explanation, who is in the room, where the sessions happen, how the family's authority structure is respected and adapt the content cautiously, because a technique that has been changed beyond recognition is no longer the intervention that was tested. Where you adapt substantially, say so in the record and in supervision, and monitor the outcome.

The professional's obligation. Families from minority or marginalised groups are more likely to receive poorer care, to be diagnosed later, to be offered less evidence-based treatment, and to disengage early. This is a documented pattern across health systems, and the therapist's response to it is not guilt but method: measure outcomes, attend to engagement, and use supervision to examine the ways in which the service is failing this family.

10.14 Six errors

1. **The cultural shortcut.** Explaining the family's behaviour by their background before hearing their account. Fix: ask first, and use culture to generate questions rather than to close them.
2. **The neutral therapist.** Refusing to notice difference, and thereby obliging the family to accommodate the therapist's comfort. Fix: name difference when it is relevant, briefly and without ceremony.
3. **The extraction session.** A session spent asking the family to educate the therapist about their traditions. Fix: general knowledge from reading and supervision; the session for this family's particulars.
4. **The cultural defence of harm.** Deferring to custom where a child or a partner is unsafe. Fix: know the law, name the concern, act transparently, and stay involved.
5. **The over-adaptation.** Rewriting an evidence-based model until it no longer resembles the intervention that was tested, and calling the result culturally sensitive. Fix: adapt engagement deliberately; keep the mechanism; record and monitor what you changed.
6. **The forgotten class.** Formulating the family's difficulty in relational terms when the difficulty is money, housing, immigration status or work. Fix: ask about the material conditions first, and let the family tell you which domain the problem belongs to.

10.15 Key terms

Term	Definition
Culturalism	Treating culture as a set of rules that predict individual families

Term	Definition
Colour-blindness	Refusing to notice cultural, ethnic or class difference and its effects
Cultural genogram	A genogram annotated with language, religion, migration, education, class, duty and stigma
Acculturation gap	The difference in rate of cultural adaptation between parents and children
Interpreter (professional)	A trained interpreter bound by confidentiality, distinct from a family member or broker
Cultural broker	A person who mediates between a family and an institution, often informally
Face-saving question	A question phrased so that the answer does not require the respondent to lose standing
Microaggression	A brief, commonplace exchange that communicates a demeaning message to a marginalised person
Structural competency	The capacity to recognise how institutions and material conditions shape clinical presentations
Cultural adaptation	The systematic modification of an intervention's engagement or content for a cultural group

10.16 Self-test

1. Name the three mistakes described in this week and give an example of each.
2. What are the seven dimensions of a cultural genogram?
3. Give three mechanisms by which class shapes what a family presents.
4. Name three sites of power in a therapy session.
5. What is the professional's position when a family's custom conflicts with the law?
6. What does the evidence say about therapist client ethnic matching?
7. What is the acculturation gap, and why does it matter clinically?
8. What are two ways a therapist can adapt engagement without distorting the model?

Answers. 1. Culturalism ("Nigerian families are hierarchical"), colour-blindness ("I do not see race"), and essentialism about the family's own account ("in our culture a wife does not contradict her husband"). 2. Migration history; languages; religion and religious change; education; occupation and class; duty and individuation; stigma and disclosure. 3. Scarcity shapes the presenting complaint; attendance is affected by cost and shifts; time is not equally available, so homework and evening conversations may be impossible. 4. Who defines the problem; who is interpreted or spoken about; whose emotions are permitted; whose account is believed; who is treated as the expert.

1. The legal and professional obligation prevails; act transparently, tell the family in advance what you are doing,

and remain involved where possible. 6. The evidence for matching alone is mixed; the apparent mechanism is the expectation of being understood, and language matching has stronger support. 7. Parents and children adapt to a new culture at different rates,

producing a family in which the young person knows the new rules better than the parents while still being expected to obey the old ones; it is a common source of presentations labelled defiance. 8. Matching language and explanation; changing who is in the room and where sessions happen; adjusting the framing to the family's values while keeping the mechanism of the intervention intact.

10.17 Reflective task

Where in your own family would a therapist from a different culture misread the structure? Write 300 words on what they would get wrong and why. Then write 100 words on what you would want them to ask you, and in what form. Bring both to the seminar; the second paragraph is usually the more useful one, because it is the question you will need to ask a family that is not yours, and in the form in which you would want to be asked it yourself, by somebody who had time to listen to the answer.

10.18 Readings for this week

- **McGoldrick, Giordano & Garcia-Preto (2005)**, Ethnicity and Family Therapy, 3rd edition read one chapter on a group you work with, and one on a group you do not, to see the difference between description and stereotype.
- **Hays (2016)**, Addressing Cultural Complexities in Practice, for the ADDRESSING framework and its clinical use.
- **Falicov (2014)**, Latino Families in Therapy the best single account of a culturally grounded family therapy method, useful well beyond its stated population.
- **Boyd-Franklin (2003)**, Black Families in Therapy, for the integration of race, class and clinical method.
- **Griner & Smith (2006)** and later meta-analyses of cultural adaptation, read for the size and character of the effect.
- **Your national code of conduct and safeguarding guidance**, read this week rather than next, because this is the week in which the conflicts become concrete. Mark the three paragraphs you would have to quote aloud to a family, and practise saying them.

Next: Week 11 What the evidence says

Week 11 What the evidence says

Students often arrive believing either that therapy "just works" or that only manualised treatments do. The evidence is more interesting than either position, and a clinician who cannot read it is dependent on whoever is willing to summarise it for them.

11.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0 10	Two bad positions	"It just works" and "only manuals work", stated and dismantled	Identify which they hold

Minutes	Segment	What the lecturer does	What students do
10 25	Designs and what they can answer	RCT, cohort, case series, qualitative, synthesis	Match six questions to six designs
25 40	Effect sizes in plain language	d, NNT, percentages, translated into families	Convert four results into "out of ten families"
40 60	The trial map	Eating disorders, conduct, substance use, psychosis, couple distress	Place each finding on a strength-of-evidence grid
60 75	Reading a paper in twenty minutes	Live walkthrough of a real abstract and results table	Apply the six questions in pairs
75 85	Feedback and outcome monitoring	The evidence for measuring	Design a session-by-session measure
85 90	Set the assignment	The critical appraisal	Choose a paper

11.2 Why a clinician needs research literacy

Four reasons, each of which will appear in an examination.

Because the family will ask. "Does this work?" is the commonest question a family asks in the first session, and an answer of "yes, therapy helps" is not an answer. The clinician needs to know what works for what, with what effect, and for whom, in order to give an honest answer that includes uncertainty.

Because services are commissioned on evidence. Funding, waiting lists and staffing follow evidence of effectiveness. A profession that cannot read the evidence cannot argue for its own resources, and family therapy has spent decades arguing from conviction rather than from data at exactly the wrong moments.

Because the alternative is fashion. Without research literacy, practice follows the last conference, the most charismatic trainer, or the model a service happened to invest in. The result is a profession in which whole counties practise one way and whole counties another, with no one able to say which is better.

Because clients are harmed by ineffective treatment. The most important finding in psychotherapy research over the last thirty years is not that therapy works; it is that some clients deteriorate in treatment, that therapists often fail to notice, and that routine feedback reduces the harm. A clinician who does not measure cannot know whether they are helping the family in front of them.

11.2a How evidence gets into a service

A student who understands the evidence but not how services change will spend a career frustrated. Four mechanisms, and the clinician's place in each.

Commissioning. Services are funded against specifications that cite guidelines, waiting times and outcome targets. A clinician who can read a specification and contribute outcome data is in a position to influence what is commissioned; a clinician who cannot is dependent on others' descriptions of what works. This is why routine measurement matters beyond the individual case: it is the profession's evidence in negotiations.

Fidelity and adaptation. Evidence-based models are usually tested with training, supervision and monitoring of fidelity the degree to which the model is delivered as designed. Services often adopt the name of a model without the training and supervision, and then report that the model does not work. The clinician's question about any new service should be: what training, what supervision, and how will fidelity be monitored?

Scale-up. Programmes that work in a research trial frequently produce smaller effects when delivered at scale: the therapists are less selected, the supervision is thinner, the population is broader, and the organisational support is weaker. Multisystemic therapy's large-scale rollouts are the standard cautionary example. This is not an argument against evidence-based practice; it is an argument for measuring outcomes continuously, in the service as delivered.

The professional's position. The clinician is not a passive recipient of policy. Three things are within reach of almost everyone: measure your own outcomes; bring them to team meetings and supervision; and learn to write a one-page summary that a manager can use. The alternative waiting for the research to be translated by somebody else is what has produced the gap between what is known and what is done.

11.3 Designs, and the questions they can answer

A design is not "better" or "worse" in the abstract; it is better or worse for a question. Six questions and the designs that answer them.

Question 1: does it work, compared with what would otherwise happen? Answer: a randomised controlled trial (RCT). Families are allocated at random to the treatment or to a comparison (waiting list, treatment as usual, another treatment). Randomisation is what allows causal claims, because it distributes unknown confounders across groups. The trial answers the question for the population and the conditions studied, and no further.

Question 2: does it work in ordinary services, with ordinary families? Answer: a pragmatic trial, or a large practice-based cohort. Efficacy (can it work under ideal conditions?) and effectiveness (does it work in the clinic on a Tuesday?) are different questions, and the gap between them is one of the recurring disappointments of the field.

Question 3: what happens over years? Answer: a cohort or a long-term follow-up of a trial sample. Attrition is the enemy; a five-year follow-up that retains 40% of the sample tells you about the 40%.

Question 4: how does it work? Answer: a mediation analysis, a process study, or a qualitative study. Mechanisms are examined by measuring the hypothesised mediator and testing whether change in it precedes change in outcome. Most family therapy models have weak mechanism evidence relative to their outcome evidence.

Question 5: what is it like to receive it? Answer: qualitative methods interviews, focus groups, thematic or narrative analysis. These cannot tell you whether a treatment works, and they are the only way to find out why families disengage, what the experience of being assessed feels like, or how a model is actually delivered.

Question 6: what does the whole body of evidence say? Answer: a systematic review with meta-analysis, or a guideline. The quality of the answer depends on the quality of the included studies, on publication bias, and on whether the reviewers looked for the studies that did not work.

The hierarchy, stated honestly. Systematic reviews of well-conducted RCTs sit at the top for questions of effect; qualitative studies sit at the top for questions of experience; a single case series is the best available evidence for a rare presentation in your clinic. Any hierarchy that ranks designs without reference to the question is a hierarchy of prestige, not of knowledge.

11.4 Effect sizes in plain language

The most useful skill in this week is converting a statistic into a sentence a family could understand.

What d means. Cohen's d expresses the difference between two groups in standard deviation units. Roughly: d = 0.2 is small, d = 0.5 moderate, d = 0.8 large. In practice, for psychotherapy, meta-analytic effects for depression and anxiety cluster around 0.5 to 0.8; couple therapy for relationship distress shows effects in the moderate-to-large range; family-based treatment for adolescent anorexia is associated with remission rates substantially higher than comparison treatments at end of treatment and follow-up.

Converting d to something a family can picture. A d of 0.5 means that the average treated family is better off than about 69% of untreated families. In a group of ten comparable families, roughly seven would do better with treatment than the average family without it. That is a meaningful benefit and it is not a cure, and saying both is the professional's obligation.

What NNT means. The number needed to treat is the number of families who must receive the treatment for one additional good outcome. An NNT of 5 means that for every five families treated, one does better than they would have done otherwise; the other four either would have improved anyway or do not benefit. Families find this framing honest and clinicians find it uncomfortable, which is a sign it is worth using.

What p-values do not tell you. A p-value is the probability of the data, or more extreme data, if the null hypothesis were true. It says nothing about the size of the effect, nothing about whether the effect matters, and nothing about the individual family. A trial with 4,000 families will produce $p < 0.001$ for an effect too small to notice, and a trial with 30 families will produce $p = 0.4$ for an effect that transforms lives. Read the effect size and the confidence interval first.

Confidence intervals, in one sentence. The 95% confidence interval is the range of effects compatible with the data; a wide interval means the study cannot tell you much, and an interval that crosses zero means the study cannot exclude no effect.

EXERCISE

Take any four results from this week's reading, and write for each: one sentence a family could understand, one sentence a commissioner could understand, and one sentence of honest limitation. The third is the one that marks you as a professional.

11.4a Reading a forest plot

The forest plot is the signature graphic of a meta-analysis and the quickest way to see a body of evidence at once.

What is on the plot. Each study is a horizontal line: the point estimate of its effect is a square (sized by the study's weight), and the line through it is the confidence interval. The vertical line at the centre is "no effect". A pooled diamond at the bottom summarises the overall estimate.

How to read it in four looks.

Look 1 where do the squares sit? If most squares are on one side of the no-effect line, the body of evidence points one way. If they are scattered across it, the evidence is inconsistent, and the pooled estimate conceals the inconsistency.

Look 2 how long are the lines? Long lines mean small studies with imprecise estimates. A plot dominated by long lines cannot support a confident conclusion, however impressive the diamond.

Look 3 how wide is the diamond? The diamond is the pooled confidence interval. Narrow means precise; wide means the meta-analysis cannot tell you much, usually because there are few studies or they disagree.

Look 4 is there an asymmetry? If small studies cluster on the positive side and large studies sit nearer the no-effect line, suspect publication bias. Funnel-plot asymmetry is the formal version of the same observation.

What the plot cannot show. Whether the included studies measured what you care about; whether the interventions were delivered as designed; whether the participants resemble your clinic; and whether the authors selected which studies to include. All four are in the text, and all four matter more than the diamond.

11.5 The map of the evidence

A brief, honest map of what family and couple therapy has, organised by presentation. Students should learn the shape of this map and the two or three key references for each entry, not memorise numbers.

Relationship distress in couples. The strongest evidence base in the field. Behavioural couple therapy, cognitive-behavioural couple therapy and emotionally focused therapy all show moderate-to-large effects on relationship satisfaction in randomised trials, with a substantial minority of couples deteriorating or not responding, and with relapse a real phenomenon over one to two years. The comparative evidence between bona fide couple therapies is weak; the alliance and the couple's readiness matter a great deal.

Adolescent behaviour problems, including conduct disorder. Family-based approaches parent management training, functional family therapy, multidimensional family therapy, multisystemic therapy have good evidence for reducing behaviour problems, with effect sizes in the moderate range, and with the strongest evidence where the intervention is intensive and delivered with fidelity. The evidence is weaker for adolescents with callous-unemotional traits and for those whose difficulties are embedded in severe neighbourhood disadvantage, where effects have been disappointing in large-scale rollouts.

Adolescent substance misuse. Multidimensional family therapy, functional family therapy and brief strategic family therapy have trial evidence for reducing use, with effects that are typically modest and often not maintained without continued support. The comparison with individual treatment is favourable; the comparison with doing nothing is more favourable still.

Eating disorders. Family-based treatment (the Maudsley approach) for adolescent anorexia nervosa is a first-line, guideline-endorsed intervention, with evidence comparable or superior to individual treatment at end of treatment and at follow-up. For bulimia, family involvement is a component of several evidence-based approaches. This is the clearest example in the field of a specific family intervention for a specific presentation, and it earns its place in the curriculum.

Psychosis. Family interventions combining psychoeducation, communication training and problem-solving reduce relapse rates in schizophrenia when added to medication, with a number needed to treat in the range of five to seven families for one relapse prevented over nine to twelve months. The evidence is old, replicated, and under-implemented, which is itself a lesson about services.

Depression. Couple therapy is an effective treatment for depression where the relationship is distressed, with effects comparable to individual therapy in several trials, and with the additional benefit of improving the relationship. Behavioural activation delivered to a couple is a legitimate and well-evidenced option.

Child and adolescent anxiety and depression. Family involvement in cognitive-behavioural treatment improves outcomes modestly; systemic family therapy has a smaller but real evidence base; the strongest evidence in this area concerns parental accommodation of anxiety, which is a family-level mechanism that can be addressed directly.

Physical health and long-term conditions. Family interventions improve adherence and outcomes in diabetes, asthma and some chronic pain conditions, with small-to-moderate effects. This is an area of growing importance in services with integrated care.

What the map teaches. Family therapy is not one treatment with one evidence base; it is a family of methods with strong evidence in some presentations, moderate in others, and thin in the areas where the profession most wants recognition (for example, complex trauma in adults, where individual and phase-based approaches dominate). Honesty about the thin areas is part of competence.

Four findings worth carrying into practice.

1. **Family and couple therapies work.** Meta-analyses consistently find moderate-to-large effects for couple therapy in relationship distress, and for family-based treatments in adolescent behaviour problems, eating disorders (family-based treatment for anorexia is a first-line intervention), substance misuse and psychosis relapse.
2. **Alliance matters, and it is relational.** In couple and family work, the alliance is not one relationship but several, and shared sense of purpose predicts outcome better than any single dyad's alliance with the therapist.
3. **Model differences are smaller than model loyalties suggest.** Common-factors research attributes much of the variance to client factors, alliance and expectancy. Technique is not irrelevant: it organises what the therapist notices and does, and it matters more for specific problems.
4. **Drop-out is the silent failure.** In couple and family work, 20 40% of families end therapy unilaterally. Attendance, engagement and early change are the best predictors of retention. Track them.

11.5a Two studies that changed practice, described properly

Learning the detail of two landmark studies does more for research literacy than learning the names of twenty.

Study 1 family therapy for adolescent anorexia nervosa (Russell and colleagues, 1987). Eighty patients with anorexia nervosa were randomised to family therapy or to individual supportive therapy after admission and refeeding. Both treatments were delivered over about a year, with family therapy beginning as a single family session and developing into conjoint family work focused on the parents taking charge of the patient's eating. The central finding was an interaction: in patients whose illness had begun before the age of 19 and whose duration was under three years, family therapy produced substantially better outcomes at one year; in patients with a longer illness, individual therapy did at least as well. The study was small, conducted in one centre by a committed team, and used a comparison treatment that critics argue was not designed as an active competitor. It also changed the treatment of adolescent anorexia nervosa permanently, and later trials and replications supported its central finding. It is a model of a study that is simultaneously underpowered by modern standards and enormously influential, because its finding was specific, plausible and clinically decisive.

Study 2 family intervention for schizophrenia relapse (Leff and colleagues, 1982, and the subsequent replications). Patients with schizophrenia living in households characterised by high expressed emotion criticism, hostility and emotional over-involvement have substantially higher relapse rates than patients in low-EE households. Leff's trial randomised patients discharged to high-EE homes to a family intervention (education, a relatives' group, and sessions at home) or to routine care. Relapse over nine months was markedly lower in the intervention group, and the finding was replicated across several countries with a consistent effect on relapse. The number

needed to treat is in the range of five to seven families for one relapse prevented. What makes this study instructive is the mechanism: the intervention did not change the illness, it changed the emotional environment in which the illness was managed. It is also instructive in another way: the intervention was never widely implemented, for reasons that have nothing to do with evidence and everything to do with service design, professional boundaries and the difficulty of working with families in acute psychiatric settings.

What the two studies teach.

First, the interaction matters. The anorexia trial's finding is conditional on age and duration of illness; a clinician who applies the result to a 35-year-old with a twelve-year history is misreading the evidence.

Second, mechanism is not optional. The expressed-emotion work identified a modifiable family process and changed it; that is a stronger scientific story than a model-level claim that family therapy works.

Third, implementation is a separate problem. A treatment can be well evidenced and unused. The gap between evidence and practice is the subject of the rest of a professional training, and it is not closed by more evidence.

11.6 Reading a paper in twenty minutes

The method, taught as a sequence, so that it can be done under time pressure in a clinic.

Minutes 1 3: the abstract, backwards. Read the conclusion first, then the results, then the method. Abstracts are written to persuade; reading backwards makes the persuasion easier to see. Ask: what did they find, and what did they do to find it?

Minutes 3 5: the design paragraph. Who was recruited, from where, with what inclusion and exclusion criteria, and how many dropped out. The exclusion criteria tell you who was not studied, which is how you decide whether the sample resembles your clinic. A trial that excluded families with active substance misuse, no fixed address, or a child under six cannot tell you about the families you most often see.

Minutes 5 8: the comparison. What was the treatment compared with? Waiting list comparisons inflate effects; treatment-as-usual comparisons vary enormously depending on what usual treatment is; head-to-head comparisons between active treatments are the most informative and the rarest.

Minutes 8 12: the outcome measure. Does the measure capture what the family cares about? Who completed it the parent, the child, the therapist, or an independent assessor? Blinded assessors matter, and in family therapy they are hard to maintain, because families mention their treatment.

Minutes 12 15: the numbers. Effect size, confidence interval, the proportion who improved, the proportion who deteriorated, and the number who dropped out. The last two are routinely omitted from abstracts and are often the most clinically important.

Minutes 15 18: the limitations. Read the authors' own limitations section, then add the two they did not mention: publication bias (are we reading this because it worked?) and allegiance (do the authors have a stake in the model they tested?).

Minutes 18 20: the verdict. Write three sentences: this study shows X, with these limits; I would use it when Y; it would not apply to Z. If you cannot write the third sentence, you have not read the paper you have read its abstract.

The six questions, condensed. Who? Compared with what? Measured how? How big? How sure? Does it fit my clinic?

11.6a Where to find the evidence quickly

A clinician does not need to read the primary literature every week; they need to know where the good summaries are and how to check them.

Source	What it is good for	Watch for
National guidelines (NICE, SIGN, APA, and local equivalents)	The current standard of care for a defined presentation	Lag; local applicability; the guideline's scope
Cochrane and Campbell reviews	Questions of effect, with explicit methods	Reviews can be out of date; check the date
Professional association guidance (for example, family therapy associations)	Competence frameworks and model-specific summaries	Allegiance
Systematic review databases and evidence maps	Finding what exists before reading anything	Coverage of non-English and non-Western literature
A good textbook, current edition	The shape of a field in a week	Lag of three to five years
A trusted colleague with a special interest	Orientation and a shortlist	The same biases as yours
The family in front of you	Whether any of it applies	Small n, and no comparison group

The habit. For each presentation you meet regularly, keep a one-page summary: the recommended first-line approach, the key trial or review, the main alternative, the main uncertainty, and the reference. Update it once a year. Five of these pages will cover most of a general family therapy caseload, and they are worth more than a shelf of unread journals.

11.7 Annotated abstract: reading between the lines

The following is a composite abstract constructed for teaching, containing the features students most often miss.

> "In a randomised controlled trial, 120 families with a child aged 11 15 referred for behavioural difficulties were allocated to family therapy (n = 60) or a waiting list (n = 60). The primary outcome was the child's behaviour score at 12 weeks, rated by parents. Family therapy was superior (mean difference 4.2 points, 95% CI 1.1 to 7.3, d = 0.42). Secondary outcomes included attendance at school. Twenty-two per cent of the family therapy group did not complete the intervention."

What the abstract tells you. There was a trial; it was randomised; the comparison was a waiting list; the outcome was parent-rated; the effect was modest; a fifth of the treated families did not complete the treatment.

What it does not tell you. Who rated the child (the parents, who are not blind to treatment). What happened to the waiting-list group afterwards (did they get the treatment, and when?). What "family therapy" consisted of (model, dose, fidelity). What happened to the school attendance outcome (it appears in the secondary list and then disappears from the abstract, which usually means it did not reach significance). Whether the improvement was maintained beyond twelve weeks.

What the numbers mean, in plain terms. A d of 0.42 means the average treated child was better off than about 66% of untreated children on the parent-rated measure. The confidence interval excludes zero, so the effect is unlikely to be chance, and it is small enough that six or seven families in ten would not have shown the benefit. The 22% dropout means that one family in five did not get the full dose, which is entirely typical and is the finding a service should act on.

The question to ask the authors. "What happened to the families who dropped out, and how does the effect look if you analyse them as treatment failures?" The answer is often in the paper's supplementary tables, and it is the answer that matters for a clinic.

11.7a Appraising a qualitative study

Qualitative research answers questions that trials cannot, and it has its own quality criteria. Five questions.

1. Is the question appropriate to the method? "How do families experience a first session in which a therapist asks about the father's drinking?" is a qualitative question. "Does asking about drinking improve outcomes?" is not.

2. Is the sampling described and justified? Who was interviewed, how were they recruited, and who declined? Families who agree to be interviewed about therapy are not a random sample of families in therapy, and the paper should acknowledge it.

3. Is the analysis transparent? Were transcripts coded systematically, by more than one person, with the codes and themes shown? Is there a "audit trail" from data to interpretation? Are negative cases discussed?

4. Does the paper stay close to the data? Good qualitative papers quote extensively and distinguish what participants said from what the authors concluded. Look for the place where the authors' interpretation outruns the quotes.

5. Is reflexivity present? Did the authors consider their own position as clinicians, as researchers, as members of the same or a different culture as the participants and its effect on what they saw?

What a good qualitative study gives a clinician. The vocabulary of the family's experience, which is otherwise invisible; the reasons for dropout, which outcome data record as a number; the details of what helps, which families describe in ways that no questionnaire anticipates. It will not tell you what to do, and it will change how you do it.

11.8 Routine outcome monitoring and feedback

The most robustly supported innovation in everyday practice is not a model; it is measurement with feedback.

What it is. Families complete a short measure every session or every few sessions the ORS and SRS, the SCORE-15 or its session-by-session short form, symptom measures where relevant. The scores are plotted, compared with an expected trajectory, and shown to the clinician and the family. Where progress is off-track, the clinician changes something.

What the evidence shows. Feedback to clinicians improves outcomes overall, with the largest benefits for clients who are not progressing or are deteriorating that is, exactly the clients a clinician's judgement is most likely to miss. Several trials find that feedback alone reduces deterioration rates; the effects on the average client are smaller.

What it looks like in a family session. Two minutes at the start: each member completes the measure; the therapist notes the scores; at the end, one question. "Your ratings went from 26 to 21 this fortnight what has been different? And yours went from 18 to 20, which suggests something has got harder; is that right?" The measure becomes a third participant, and families often say things to the measure that they would not say to the therapist.

The discipline required. Plot the scores; do not merely file them. Use the scores in supervision. Have an agreed response to deterioration, which should include a review of the formulation rather than a repetition of the same intervention. And never use the measure as a test the family can fail.

The limits. Measures are culturally loaded, they privilege the presenting symptom, and they cannot capture what matters most to many families. They are an instrument, not a verdict and an instrument in the hands of a clinician who also listens.

11.8a Choosing measures for a family clinic

A small, workable measurement set, with the trade-offs stated.

The session-by-session pair. The Outcome Rating Scale and Session Rating Scale take under two minutes each, are free for individual use in many jurisdictions, and produce a plot the family can see. Their weakness is that they are very short and give no detail about what changed.

The family-functioning measure. The SCORE-15 gives fifteen items and several perspectives, including the family's own account of strengths and difficulties, and it has been used widely in UK family therapy services. It is completed at assessment, mid-point and end, rather than every session.

The symptom measure. Where the presentation is a defined condition depression, anxiety, eating disorder, conduct problems use the standard measure for that condition, so that your outcomes are comparable with the literature and with other services.

The family's own measure. Two or three statements written by the family, scored weekly. This is the measure that families find most meaningful, and the one that most often captures the change the standard measures miss.

The rule. Two minutes per session, one fuller measure at assessment and review, one family-defined target, and a plot that is shown to the family. Anything more elaborate will be abandoned within a month; anything less will leave you guessing.

11.9 Common factors, specific factors, and the therapist

Three findings about how therapy works, each contested and each useful.

The common-factors position. Client factors, the alliance, the therapist's empathy and the family's expectancy account for a large share of outcome variance; model differences account for a smaller share. The position is supported by comparative trials that find few differences between bona fide treatments, and by meta-analytic work on alliance.

The specific-factors position. Technique matters, particularly for specific presentations: exposure for phobias, family-based treatment for adolescent anorexia, behavioural methods for conduct problems, communication and problem-solving training for expressed emotion in psychosis. The evidence for specificity is strongest where the disorder is well defined and the mechanism is well understood.

The therapist-effects position. The most uncomfortable finding in the field: therapists differ substantially in their outcomes, and the differences are not explained by training, experience or theoretical orientation. Some therapists reliably produce better outcomes with the same clients and the same model; some reliably produce worse, and appear unaware of it. Deliberate practice with feedback is the current best candidate for improving this.

What a trainee should take from all three. Work on the alliance, because it is the factor you can influence session by session; learn the specific methods that have evidence for the presentations you meet, because they organise your attention and your actions; and measure your own outcomes, because you cannot otherwise know which therapist you are.

> **DEBATE**
> If therapist effects are larger than model effects, what is the point of learning a model? Defend your answer with reference to the evidence.

11.9a The replication problem, and what it means for a trainee

Several of the most celebrated findings in psychology and psychotherapy have proved harder to replicate than their original reports suggested, and the field has responded with pre-registration, larger samples and a harder look at statistical practice.

Why it matters clinically. A trainee who learns a technique from a single impressive study may be learning an artefact. The defences are simple: prefer systematic reviews to single studies; prefer large samples to small; read the effect size and the confidence interval rather than the p-value; and be suspicious of any result that arrives with a marketing campaign attached.

Why it does not license nihilism. Replication problems are concentrated in social psychology and in some individual-difference research. The core findings of psychotherapy outcome research that therapy works, that the alliance matters, that family interventions help in the presentations described in 11.5 are supported by multiple independent trials, by different research groups, and by practice-based data. The evidence is uneven, not empty.

The professional's habit. Ask of any striking claim: has it been found by someone other than the people who invented the method? That single question will protect you from most of the field's enthusiasms, and it is the reason this week's readings include critics of the models we teach.

11.9b Two measures of the alliance, and how to use them

The Session Rating Scale and the Revised Helping Alliance Questionnaire both measure the therapeutic relationship; the first is four visual-analogue items, the second is longer and includes goal and task agreement. In family work the therapist should consider three perspectives rather than one.

Who to ask. The adult who organised the referral, the adult who did not, the child over about nine, and where relevant the adolescent who has been brought rather than consulted. A family in which the mother rates the alliance at 34/40 and the father at 16/40 is a family in which one member is likely to drop out, and the rating tells you which.

When to ask. Every session for the first three, then every third. Two minutes at the end of the session, with the family completing it in the room.

What to do with a low score. Not to reassure. The useful sequence is: thank the member; ask what would have made the session more useful; ask what the therapist did that made it less useful; and change one thing next time, visibly. Families notice when a therapist acts on a low rating, and the act of doing so frequently raises the rating more than any technique.

What to record. The score and the action. "Father 16/40 at session two; said he felt 'talked over'; agreed that I would ask him first at the start of each session. 29/40 at session three." That sentence, in a supervision file, is worth more than a page of impressions.

The caution. A high alliance score is not evidence of good therapy; families can be warmly engaged in work that is not helping. Measure outcome alongside alliance, and when the two disagree, trust the outcome and interrogate the alliance.

11.10 The family's own evidence

Research evidence is population evidence; it is not evidence about this family. Three ways to build the family's own data into practice.

Single-case designs. A baseline, an intervention, a return or a comparison, and repeated measurement across all three. In a family setting, a two-week baseline of the target behaviour, followed by a specific intervention, with the family rating the target weekly, is a legitimate and under-used method. It answers the question the family is actually asking: "is this working for us?"

Idiographic measures. Ask the family at the outset to write the two or three statements that would tell them things had improved "we would eat together twice a week", "I would ask once and she would answer". Score them weekly. The measure is specific to the family, which is its point.

Practice-based evidence. A service that routinely collects outcome data can compare its own results with published benchmarks, identify which presentations it serves well and which it does not, and adjust. This is how a clinic becomes a learning organisation rather than a place where fashions arrive from outside.

The professional's stance. "The research tells me that about seven families in ten with this presentation benefit from this kind of work. I do not know yet whether you are one of them; I would like us to measure it together, and review in six weeks." That sentence contains population evidence, uncertainty, and a commitment to the family's own data, and it is the shape of an evidence-informed practice.

11.10a The one-page evidence sheet

The practical output of this week is a personal evidence sheet for one presentation you meet regularly. The format is deliberately short.

Heading: the presentation, and the age group.

What is recommended first-line, and by whom. One or two sentences, with the guideline and its date.

The key trial or review, in three sentences. What was done, what was found, and how strong the finding is.

The main alternative, and when to choose it. For example: individual therapy where the family cannot be engaged; a supported-carer model where the parents cannot take the required role.

The main uncertainty. What the evidence does not settle.

What I will measure, and when I will review. The measure, the interval, and the criterion for changing course.

One reference, and one person I can ask.

Why the sheet rather than a reading list. Because a clinician under pressure will consult a page, not a literature. Because writing the sheet forces the four judgements that matter what is first-line, what is the alternative, what is uncertain, and what will I measure and because it is revisable when the guideline changes or when your own data disagree with it.

> **TASK**
> Complete one evidence sheet for a presentation you have seen in placement. Bring it to the seminar and exchange with a peer working with a different presentation. Then answer: which of the two sheets would you trust if you were the family?

11.11 Six errors in using evidence

1. **Overclaiming.** "Family therapy is proven for depression" when the evidence concerns couple therapy for depression in distressed relationships, in adults, in trials of twelve to twenty sessions. Fix: say what the evidence covers, and name the boundary.
2. **Cherry-picking.** Citing the trial that supports the model and omitting the two that did not. Fix: when you cite evidence for a method, name the best study against it and say why you still use the method.
3. **Dismissing evidence because it is inconvenient.** "That trial used a manual, and real families are not like that." Sometimes a legitimate objection, usually a defence. Fix: write down the specific reason the evidence does not apply, and check whether the same reason would excuse a method you dislike.
4. **Confusing the model with the evidence.** Assuming that because family-based treatment works for adolescent anorexia, family therapy works for everything, or that because one trial failed, the approach is worthless. Fix: keep the map of the evidence by presentation, and consult it before making claims.
5. **Ignoring dropout.** Reporting outcomes only for completers, or planning a service on the assumption that families attend. Fix: treat attendance as an outcome, and measure it.
6. **Evidence without measurement.** Reading the literature diligently and never measuring your own results. Fix: the family's own data, session by session, is the minimum standard of an evidence-informed clinician.

11.11a The journal club that works

Most journal clubs fail: one person presents, everybody nods, nobody changes anything. A workable format, used in several training programmes, takes forty-five minutes and changes practice.

Before (10 minutes). Everyone reads the abstract and the results table only. Everyone writes down, in one sentence, what the study found and one question they would ask the authors.

Presentation (10 minutes). One person presents using the six questions of 11.6 and is not allowed to speak for more than ten minutes. The discipline is the point: a paper cannot be presented properly in ten minutes unless it has been understood.

The clinic test (10 minutes). Each participant names one family on their caseload to whom the study would or would not apply, and says why. This step is what converts reading into clinical reasoning, and it is the step most clubs omit.

The disagreement (10 minutes). The presenter must argue the strongest case against the study's conclusion, and somebody else must defend the study. The roles are assigned, not chosen.

The action (5 minutes). One change to practice, however small, with a date: a measure to be added, a referral criterion to be reviewed, a family to be discussed in supervision. The change is recorded, and at the next journal club the first item is whether it happened.

Why this works. Because it is short, because it requires reading, because it forces the clinic test, and because it ends in a change rather than in agreement. A team that runs this monthly will accumulate twelve practice changes a year, which is more than most services achieve in a decade of training days.

11.12 Culture, faith and the evidence

The evidence base is not culturally neutral, and the critique is now well developed.

Who was in the trials. Most psychotherapy trials are conducted in high-income countries, with samples that are disproportionately white, educated, urban and partnered. The extent to which findings generalise to families in Ibadan, to families with strong religious commitments, or to families in multigenerational households is an empirical question that has not been adequately studied. The clinician's obligation is to apply the evidence thoughtfully and to monitor the outcome, not to assume either that it applies or that it does not.

What counts as an outcome. Trials privilege symptom scores and relationship satisfaction, both measured with instruments developed in the same settings. A family may care about whether the household is peaceful, whether the children are respectful, whether the family's standing is intact, or whether a debt has been paid. The movement for culturally grounded and locally defined outcomes is a corrective, and it has not yet changed the main evidence base.

The cultural validity of measures. A questionnaire translated into another language is not necessarily measuring the same construct; a scale developed to assess "family functioning" in one setting may describe a different construct in another. Where measures are used across cultures, they should be validated locally, and the clinician should ask what the item means to this family rather than assuming.

Faith and the evidence. Where families hold spiritual explanations, the evidence base for collaborative approaches working with religious coping rather than against it, and integrating spiritual and psychological frameworks with consent is small but encouraging, particularly in depression and substance use. The relevant professional skill

is not to adjudicate between frameworks but to work with the family's own resources while remaining honest about what the evidence supports.

> **AUDIT**
> Take any guideline you use. Count the studies cited from outside North America and Europe. Then ask what that count implies for a clinic in Ibadan, and write 150 words to a colleague explaining how you will use the guideline anyway.

11.13 Evidence note: where the field is genuinely contested

An honest account of four live disagreements, because students should know that the field is not settled and that competent clinicians disagree.

Contest 1: family-based treatment versus individual treatment for adolescent anorexia. Family-based treatment has strong trial support and is recommended first-line in several guidelines. Critics argue that the trials are small, that the comparison treatments were not always optimally delivered, that the model's emphasis on parental control of eating is difficult in some family configurations, and that newer approaches (for example, supported carer models and adolescent-focused therapy) perform comparably in some trials. The practical position is that family-based treatment is a strong first-line option and not the only defensible one.

Contest 2: do model differences matter? Comparative trials frequently find small or no differences between bona fide therapies, and common-factors researchers argue that this reflects the dominance of shared mechanisms. Others point to specific effects in specific conditions, to the poor quality of many comparative trials, and to the fact that "no difference" in a small trial is not evidence of equivalence. The practical position: model matters most for specific presentations; alliance and feedback matter everywhere.

Contest 3: early intervention and prevention. Programmes for couple relationship education and for preventing child behaviour problems show small effects in universal populations and larger effects in targeted, higher-risk groups. Whether the small universal effects justify the cost is a policy question, not a clinical one, and clinicians should be careful not to overstate prevention claims.

Contest 4: the feedback effect. The routine outcome monitoring literature is generally positive, with some large trials finding smaller effects than the early studies and some finding none once clinician training and attention are controlled. The most defensible claim is that feedback helps most for clients who are not progressing, and that the mechanism is the clinician's attention rather than the numbers.

What to do with the contested areas. State the uncertainty when you teach or when a family asks; use the intervention that has the best evidence and fits the family; measure the result; and do not let loyalty to a model substitute for attention to the evidence.

11.14 Key terms

Term	Definition
Efficacy	Whether an intervention works under ideal, controlled conditions
Effectiveness	Whether an intervention works in ordinary services with ordinary clients
Randomisation	Allocation by chance, which distributes confounders across groups
Effect size (d)	The difference between groups in standard deviation units
Number needed to treat	How many clients must be treated for one additional good outcome
Confidence interval	The range of effects compatible with the data at a given probability
Mediator	A variable through which an effect is transmitted
Moderator	A variable that changes the size of an effect for different groups
Publication bias	The tendency for positive findings to be published and negative ones not
Allegiance effect	The tendency for researchers' commitment to a model to inflate its results
Fidelity	The degree to which an intervention is delivered as designed
Routine outcome monitoring	Session-by-session measurement of progress with feedback to the clinician
Practice-based evidence	Evidence generated from routine services rather than controlled trials
Idiographic measure	A measure constructed for, and about, one family

11.15 Self-test

1. What question does an RCT answer, and what question does it not answer?
2. What does d = 0.5 mean in terms of families?
3. Why is the number needed to treat a useful statistic for a clinician?
4. What six questions should you ask of any paper?
5. What does the evidence for routine outcome monitoring show, and for whom is the effect largest?
6. Name the presentation with the strongest evidence for a specific family-based treatment, and name the treatment.
7. What is the therapist-effects finding, and why is it uncomfortable?
8. Name two ways in which the evidence base is culturally limited.

Answers. 1. It answers whether the intervention causes better outcomes than the comparison in the population studied; it does not answer whether it will work in your clinic, with your families, or by what mechanism. 2. The average treated family is better off than about 69% of untreated families roughly seven in ten. 3. It translates the effect into the number of families who must be treated for one additional benefit, which is what a service and a family need to know. 4. Who was studied? Compared with what? Measured how? How big is the effect? How confident can we be? Does it fit my clinic? 5. Feedback improves outcomes, with the largest benefit for clients who are not progressing or are deteriorating. 6. Adolescent anorexia nervosa; family-based treatment (the Maudsley approach). 7. Therapists differ substantially in outcomes, and the differences are not

explained by training, experience or orientation; it is uncomfortable because it implies that some of us are less effective and do not know it. 8. Samples are drawn mainly from high-income countries and from particular social groups; outcomes and measures reflect the priorities of those settings rather than those of the families many clinicians serve.

11.16 Reflective task

Choose one belief about your own practice that you hold strongly perhaps that a particular model is superior, or that diagnosis is harmful, or that manualised treatment cannot work with complex families. Find the strongest evidence against your belief, and write 400 words summarising it fairly. Then write 150 words on what would have to be true for you to change your practice. Bring both to the seminar; the second paragraph is the one that matters, and most students find it easier to write than they expected.

11.17 Readings for this week

- **Carr (2019 or latest edition)**, Family Therapy: Concepts, Process and Practice, the chapters on research and on evidence-based practice, for the map.
- **Stratton (2020)**, Formulation in Family Therapy, and his writing on routine outcome measurement, for the practice-based evidence argument.
- **Wampold & Imel (2015)**, The Great Psychotherapy Debate, for the common-factors case, read critically.
- **Norcross & Lambert** on psychotherapy relationships and on feedback, for the alliance and monitoring evidence.
- **The relevant NICE guideline** (or your national equivalent) for at least one presentation you meet, read as a document with a method rather than as a list of recommendations.
- **One Cochrane review** on a family intervention, read with the six questions from 11.6 in front of you.

Next: Week 12 Ethics, risk and the limits of the systemic frame

---

Week 12 Ethics, risk and the limits of the systemic frame

Ethics is not a chapter at the end of the course; it is the frame that makes the work possible. Every family therapist meets the four issues below in the first month, and meets them again every year thereafter.

12.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0 10	Why systemic thinking creates ethical problems	Four ways the frame can be used to avoid responsibility	Identify the four in a case
10 25	Confidentiality in a multiperson system	Write a confidentiality policy on the board	Draft the three sentences they will say aloud

Minutes	Segment	What the lecturer does	What students do
25 45	Risk: screening, assessing, managing	The screen, the assessment and the plan, demonstrated	Screen a role-played case in eight minutes
45 60	The limit case: secrets	Work the affair case in groups, then compare	Decide, and defend the decision
60 72	Competence and the duty to refer	What competence means when the model is relational	Audit one's own limits honestly
72 85	Dual relationships in small communities	The test, applied to five scenarios	Score and justify
85 90	Documentation	Read a good note and a bad note	Rewrite the bad one

12.2 Four ways the systemic frame can be misused

Systemic thinking is a powerful set of ideas, and like any powerful set it can be used to avoid responsibility. Four recognisable abuses, each of which a trainee will hear from an experienced colleague at some point.

Abuse 1 "The problem is in the system, not in the person", used to avoid individual responsibility. Circular causality explains how a pattern is maintained; it does not dissolve a man's responsibility for violence, a parent's responsibility for a child's safety, or a professional's responsibility for a decision. The circular account is a description of a mechanism, not a moral verdict. Where a therapist uses circularity to soften a disclosure of harm, the theory has become a defence.

Abuse 2 "We must not take sides", used to avoid protecting the weaker party. Neutrality is a technique for keeping both members engaged, not a moral position. A therapist who is neutral between a parent and a child about whether the child is safe has mistaken a stance for a value.

Abuse 3 "The family did not engage", used to avoid examining the service. Disengagement is described in the literature as a client characteristic and is often a service characteristic: waiting times, location, language, letters, the room, the first ten minutes. The ethical version of the sentence is: "the family did not return, and here is what we did that may have contributed."

Abuse 4 "It is cultural", used to avoid acting. Discussed in Week 10, and stated here as an ethical principle: culture explains, law and the code of conduct oblige.

The test. For any principle you are applying, ask: who benefits from my applying it this way in this case? If the answer is the therapist's comfort, the principle is being used as a defence.

12.2a Four principles, applied to a family

The four-principles framework respect for autonomy, non-maleficence, beneficence and justice is usually taught for individual work and becomes more interesting, and harder, when there are three or four clients in the room.

Autonomy. Whose autonomy? A father's autonomy to refuse a conversation about his drinking; a mother's autonomy to decide what is disclosed to her family; an adolescent's autonomy over their own body and information; a child's developing autonomy, which is not the same as an adult's. The principles conflict directly in family work, and no framework resolves them. What a framework does is force the therapist to name whose autonomy is being weighed, and why.

Non-maleficence. The duty not to harm. In family work the specific harms to watch for are: a disclosure that produces violence at home; a session that unites the parents against a child; a formulation that becomes a family's permanent story about one member; a record that follows a young person into adulthood. Each is a foreseeable harm and each is preventable with method.

Beneficence. The duty to benefit. In family work this includes the duty to notice when the therapy is not benefiting the family, and to change it which requires measurement, and which is why Week 11 sits beside Week 12.

Justice. The fair distribution of the therapist's time, attention and access. In a family, the therapist will be tempted to give more to the member who is more articulate, more distressed or more likeable. In a service, the therapist will be tempted to give more to the families who are easier to help. Both are justice problems, and both are visible in the record if the record is honest.

The practical use of the framework. When a decision is hard, write the four headings and, under each, the considerations in this case. Then make the decision and record the reasoning. The framework's value is not that it produces the answer; it is that it prevents the therapist from deciding on the basis of the one consideration that happened to occur to them.

12.3 Confidentiality in a multiperson system

There is no single confidentiality in family work, and the failure to say so at the start is the commonest ethical error in the field.

What to say in the first session, in three sentences. "What is said in this room, with all of us here, is shared between us. If one of you tells me something alone, I will ask you to tell the others, and I may need to raise it myself if it affects someone's safety. What you tell me is confidential outside this family, except where the law requires me to act if someone is at risk of serious harm, or a child is being harmed, I will have to tell someone, and I will tell you first that I am doing it."

Write it down. A one-page policy, given to the family at the first session, containing: who is seen together and who separately; what happens to information from individual sessions; the limits of confidentiality; how records are kept and who can see them; what happens if the family separates, or if one member asks for the record; and the complaints procedure. This page prevents most of the disputes that would otherwise occupy sessions.

The three categories of information. Useful to teach explicitly.

Shared information said in a joint session, held by the family as a whole. Can be referred to freely.

Individual information said to the therapist alone. Cannot be kept indefinitely if it bears on the work. The therapist's task is to help the member bring it into the open, with a deadline and support, and to say in advance what will happen if they do not.

Unkeepable information disclosure of risk to a child, of serious harm to self or others, of an offence that the law requires be reported. Cannot be kept, must be handled transparently, and the member should be told what will happen and when.

Secrets: a decision model. When a member discloses something and asks that it be kept, five questions in order.

1. Is anyone at risk? If yes, the secret cannot be kept, and the process of disclosure is managed, not optional.
2. Does the information bear on the work? A secret that changes the formulation cannot be held without the therapist working dishonestly.
3. What is the likely consequence of disclosure, for each member? Violence, a breakdown of the therapy, a rupture in the family, a lost job. These are weighed, not dismissed.
4. Can the member be supported to disclose? Almost always the best route: a deadline, a rehearsal, the therapist present.
5. What will the therapist do, and by when, if the member does not? This must be stated in advance, in specific terms, and honoured.

> **CASE**
> During an individual session, a husband tells you he has resumed an affair and asks you not to tell his wife. He is not at risk and nobody is being harmed in the sense that requires disclosure. What do you do? Work it in groups, then compare with the decision model above. The defensible answers differ, and each must be able to answer question 2 honestly.

12.4 Risk: the screen, the assessment, the plan

Risk is not a topic; it is a routine. Every new case, every session where something has changed, and every session before a break.

The screen (two minutes). Five questions, asked of the relevant members, in plain language, every new case.

> "In the last month, have you had thoughts of ending your life, or of harming yourself?" "Have you ever tried?" "Is anyone in the house being hurt hit, pushed, threatened, or made afraid?" "Is there anyone you are afraid of?" "Is there anything happening to the children that worries you, that we have not talked about?"

The assessment (when the screen is positive). Intent, plan, means, timeframe, protective factors, previous attempts, substance use, and the response of the people around the person. For violence: frequency, severity, escalation, weapons, children's

exposure, and the victim's own safety plan. For child protection: the specific incident, the child's account, the marks, and the family's explanation.

The plan (what must be written). Who is at risk, of what, how imminent; what has been done; who has been informed; what the family has agreed to do; what the therapist will do and by when; the next contact; and the review date. A risk assessment without a plan is a note, not an assessment.

The limits of risk prediction. Actuarial tools outperform clinical judgement for some outcomes, and none of them predict individual events well. The professional standard is therefore not accurate prediction but systematic enquiry, documented reasoning, appropriate action and review. A therapist who asks, records, acts and reviews has met the standard; a therapist who relies on a feeling has not.

The systemic dimension. Risk in families is relational: the risk to a woman is affected by who knows, who believes her, and what the family will do; the risk to a child is affected by the parents' capacity to protect, which is affected by their own circumstances. The formulation of Week 9 should include the risk, and the plan should address the relationships that make protection possible or impossible.

> **PRACTICAL**
> In pairs, take a risk history in eight minutes using the five screening questions, then write a risk plan in six lines. Swap and critique: does the plan name who, what, by when, and who else knows?

12.4b Domestic abuse: a screen that changes the work

Family work with domestic abuse is the ethical terrain on which trainees most often come unstuck, because the instincts of the model bring everyone together, hold both accounts, avoid taking sides can be exactly wrong.

The screen. Ask each adult separately, at least once, in words that do not require disclosure in front of a partner: "Has anyone in the house ever hurt you, or made you afraid? Has there been pushing, hitting, or threats?" Asking in front of the partner is not a screen; it is a test of the partner's reaction, and it can be dangerous.

The prevalence. In clinical populations, rates of domestic abuse disclosed to services are high, and in couple therapy samples substantial minorities of couples report aggression, sometimes mutual, often not. The therapist who does not ask will work with several such couples without knowing, which is both unsafe and clinically ineffective.

When disclosure occurs, four decisions.

One: is the couple work safe? Couple therapy is contraindicated where there is coercion and control, fear of the partner, escalation, or a risk of serious injury. It may be possible where there is situational aggression without control and both partners want to address it, with explicit agreements and safety planning. The distinction is not the presence of an argument; it is the presence of control and fear.

Two: what does the victim want? Not as a veto over safety, but as essential information. Many women do not want to involve the police, for reasons that are usually rational: money, children, family pressure, immigration status, and the probability of retaliation. The therapist's task is to widen the options, not to override the choice.

Three: what is the safety plan? Practical and specific a bag, a telephone, a code word, a relative, a refuge number, a copy of documents. Written, and held where the partner cannot find it.

Four: what is the therapist's legal position? In many jurisdictions, disclosure of a risk to a child triggers a safeguarding duty; some jurisdictions have specific domestic abuse reporting duties. Know yours precisely, and say what you know to the person concerned.

The error to avoid. Splitting the couple work into "communication training" while the abuse continues, which treats a safety problem as a skills problem. The formulation must include the abuse as a fact about power, not only as a pattern of interaction.

The systemic nuance worth teaching. Abuse is not symmetrical, and circular accounts that describe "mutual escalation" can obscure who is afraid. It is legitimate to describe the interaction without implying equal responsibility, and the therapist should be able to say both: "you both shout, and only one of you is afraid of what will happen next."

12.5 Competence, and the duty to refer

Taking a case you are not trained for is not courage. It is a risk to the client and to you.

What competence means. The combination of knowledge, skills, supervision and judgement sufficient to work with this presentation, this family, in this setting. It is specific: a competent family therapist may be incompetent with a family in which a child is psychotic, or with a couple where there is a live court case about custody.

Four domains in which trainees most often over-reach. Adult mental illness (where the family work must be alongside, not instead of, psychiatric care); child protection (where the therapist is not the investigator); substance misuse and detoxification (where medical management is required); and legal proceedings (where the therapist may be asked to write a report beyond their competence).

The duty to refer, and the duty not to abandon. Where a case is beyond your competence, the professional response is: discuss in supervision; identify a colleague or service with the competence; make the referral with the family's agreement; and, where appropriate, continue to work alongside. The failure mode is the therapist who refers and disappears, leaving the family to negotiate a new system alone.

The duty to develop competence. The other side of the same coin: a therapist who refers every family with a particular presentation will never become competent with it. A training plan, with supervision, is the professional way to expand the boundary.

12.6 Dual relationships and power in small communities

Small communities, religious networks and family ties make dual relationships likely rather than exceptional. The test is not "is there a second relationship?" but "does the second relationship impair the first, or exploit the first?"

The five scenarios students most often get wrong.

1. The family in your church. You will see them on Sunday. Not automatically disqualifying; requires explicit agreement about what will be said in public, and a plan for what happens if the therapy becomes known.
2. Your cousin's neighbour. A distant social connection, but information will travel. Ask the family whether they know you, and record the answer.
3. A former client's sister. Usually workable with explicit acknowledgement; the previous client's confidentiality is the constraint.
4. A family where you have already given advice informally at a family gathering. The informal relationship has already compromised the clinical one; refer, and say why.
5. A family who ask you to attend a ceremony, give a lecture to their church, or invest in their business. Each converts a clinical relationship into another kind, and each needs to be declined or examined with supervision.

The power analysis to complete. For each potential dual relationship, ask: who benefits; what could the family feel unable to refuse; what would the family's neighbours think; and what would you write in the record if a complaint were made? If the fourth answer is uncomfortable, the answer to the case is probably no.

12.7 Documentation, and the record as an ethical object

Write notes as if a colleague will take over tomorrow and a court may read them in a year: date, attendees, presenting issues, risk screen result, interventions, plan, and who was told what.

Five disciplines.

1. **Separate fact from inference.** "Mother reports two incidents of pushing in March" is a fact. "Mother is frightened of father" may be an inference; say so if you write it.
2. **Record what you did, not only what you thought.** The intervention, the question you asked, the advice you gave, the referral you made, the time you made it.
3. **Record what the family said, in their words, where it matters.** Quotation is more defensible than summary and more useful to a colleague.
4. **Record the decisions and their reasons.** "Risk assessed as low because X, Y, Z; decided not to refer; will review in two weeks." Fourteen words that would answer most of the questions a review would ask.
5. **Never write anything you would not say to the family.** The record can be requested; the phrase "manipulative mother" in a file will be read by the mother one day, and it will be indefensible.

The record and the family. Increasingly, families have a right of access to their records. Write with that in mind: the record should be truthful, respectful, and comprehensible to an intelligent non-clinician. Where you must record something painful, record it in neutral language and record the context that makes sense of it.

> **AUDIT**
> Take the last note you wrote. Underline every sentence that is inference, every adjective about a person, and every claim you could not evidence. Rewrite the note so that nothing is underlined, without losing any clinical information.

12.7b Who owns the record?

A question that arises in every family case and is almost never discussed with the family until it becomes a dispute.

The legal position varies, and you must know yours. In many jurisdictions the record belongs to the organisation that created it, and the client has a right of access, subject to exemptions where access would cause serious harm to the client or another person. In others the client owns the record. In private practice the practitioner is usually the data controller and must comply with data protection law.

The family complexity. When several people are seen, the record contains information about each of them. A request from one member raises the question of the others' rights, and the professional response is to seek the guidance of the organisation's information governance lead rather than to decide alone.

What to tell the family. In the written policy: who holds the record, who has access, for how long, how it is shared with other professionals, and how to request a copy. The families who are told this at the outset are the families who complain least, because the surprise of discovering a shared record is worse than the requirement.

What this means for writing. Write every sentence on the assumption that every member may one day read the whole document. That single discipline improves record quality more than any template, because it removes exactly the kind of sentence that should never be written.

12.8 The limits of the systemic frame

The course teaches a systemic frame because it is powerful, and a professional education owes students an honest account of what the frame cannot do.

Limit 1 it cannot substitute for individual treatment where individual treatment is indicated. Severe anorexia in an adult, psychosis, acute suicidality, a trauma requiring a phased individual approach: family work may be part of the plan and it is not the plan.

Limit 2 it cannot adjudicate facts. Whether a child was hit, whether an assault occurred, whether a man accessed an image: these are questions for investigation, and a

family therapist who offers a relational account of a disputed fact may inadvertently obscure it.

Limit 3 it can silence the individual. A focus on the system can make it difficult for a member to say "this happened to me". The therapist must be able to move between the relational and the individual register, and to say which register they are in.

Limit 4 it can become a totalising theory. When every difficulty is a system, the therapist loses the capacity to notice biology, trauma, poverty, injustice or chance. The mature position is that the systemic frame is one of several and that its usefulness is conditional on the question.

Limit 5 it can over-claim its evidence base. Week 11 sets out what is known. The ethical obligation is to practise within the evidence and to say plainly where it is thin.

The integration. A clinician who can say "I am working systemically here because the maintenance pattern is relational, and I am recommending individual trauma work for the mother in parallel" is demonstrating the frame's mature use, which is to know what it is for. The immature use is to treat the frame as a commitment rather than a tool; the mature use is to be able to name, in any given session, which frame is operating, why, and what it costs the family to have that frame rather than another.

12.9 Six ethical errors

1. **The unstated confidentiality policy.** Working for weeks and only then discovering that members assumed different rules. Fix: three sentences in the first session, written down.
2. **The secret kept too long.** A disclosure held alone while the therapy proceeds on a false basis. Fix: a deadline, support to disclose, and a stated consequence.
3. **The omitted risk screen.** Assuming somebody else asked. Fix: ask, record, and ask again after any change.
4. **The competence overreach.** Accepting a case to avoid appearing inadequate. Fix: supervision before the second session, not after the tenth.
5. **The judgmental record.** Adjectives about people, in a document that outlasts the therapy. Fix: write as if the family will read it, because one day they may.
6. **The theory used as a defence.** Circularity, neutrality or culture invoked to avoid acting on harm. Fix: ask who benefits from the principle as applied, and say the answer out loud in supervision, where it can be examined by somebody who is not invested in your comfort.

12.9a Case: the secret that involves an institution

A case that requires the confidentiality policy, the risk framework and the systemic frame to be used together.

The facts. A mother attends alone and discloses that her 15-year-old son has been taking money from her purse and that she believes he has been involved with a group of older boys who are "doing drugs". She has not told the father, who she says "would beat him".

She has not told the school, which has a policy that would lead to exclusion. She asks the therapist to keep all of it.

Why this is not a simple secret. Three of the three categories are in play. The information bears directly on the work (it is the presenting problem). Some of it is unkeepable (a child involved with drugs and at risk of harm, and a foreseeable risk of physical punishment if the father learns abruptly). And the mother's request is reasonable in her own terms: she is protecting her son from two systems she believes will harm him.

The decision model, applied.

Risk: is anyone at risk? Yes the son, from the drug involvement and from the father's likely response. Is the risk imminent? Needs assessment, not assumption.

Bearing on the work: complete. The therapy cannot proceed as though the therapist did not know.

Consequences of disclosure: to the son, a beating if the father learns without preparation; to the mother, the loss of her only confidential ally; to the family, possibly a school exclusion.

Support to disclose: the mother can be helped to tell the father in a session, with the therapist present, in a conversation that is prepared; the school conversation can be staged, with the mother and the son, focusing on support rather than sanction.

What the therapist will do if she does not: stated in advance, and specific an assessment of the son's safety with the mother's knowledge, a safeguarding consultation if the risk is confirmed, and a refusal to continue family sessions on a false basis.

The plan that follows. A session with the mother and son to assess; a joint session with both parents in which the mother discloses with the therapist's support and the father's response is managed; a school meeting arranged with the mother and son and framed around support; a safety plan for the son's contact with the older boys; and a confidentiality addendum recording exactly what was agreed and what the therapist said she would do.

What makes this ethical rather than merely competent. The therapist has not overridden the mother; she has refused to be complicit in a silence that puts the son at risk, and she has used her position to make the disclosures happen in the least harmful order. That combination honesty about the limits of confidentiality, support for the person disclosing, and action in the interest of the person at risk is the shape of ethical practice in family work.

> **DEBATE**
> The mother says: "If you tell his father, he will beat him, and that will be your fault." How do you answer? Write the three sentences you would actually say, and defend them against a peer who argues that the therapist should respect the mother's judgement.

12.10 Key terms

Term	Definition
Confidentiality (multiperson)	The negotiated rules governing what is shared, what is individual and what cannot be kept
Unkeepable information	Information that must be disclosed because of risk or law
Risk screen	A routine set of questions asked of every new case
Risk formulation	An account of who is at risk, of what, and what makes it more or less likely
Duty to refer	The obligation to involve a more competent professional or service
Dual relationship	A second relationship with a client or family member that may impair or exploit the clinical one
Competence (professional)	Knowledge, skill, supervision and judgement sufficient for this presentation, family and setting
Record access	The client's or family's right to see their clinical record
Mandated work	Therapy required by a court or statutory body
Whistleblowing	Raising a concern about unsafe practice within or outside an organisation

12.11 Self-test

1. Give two ways in which circular thinking can be misused.
2. What three sentences should introduce confidentiality in a first session?
3. What are the three categories of information in family work?
4. What is the difference between a risk screen and a risk assessment?
5. What must a risk plan contain?
6. What is the test for whether a dual relationship is acceptable?
7. Name three things a clinical note should record, and one thing it should not.
8. Name two limits of the systemic frame.

Answers. 1. To soften a disclosure of harm ("so you both escalate"), and to avoid protecting the weaker party in the name of neutrality. 2. What is said together is shared; what is said alone will be brought into the open with support; confidentiality is limited by risk and law, and the family will be told before action is taken. 3. Shared, individual and unkeepable. 4. The screen is five routine questions asked of everyone; the assessment is the structured enquiry that follows a positive screen. 5. Who is at risk, of what, how imminent; what has been done; who has been informed; what the family has agreed; what the therapist will do and by when; next contact and review date.

1. Whether the second relationship impairs or exploits the first operationally, who benefits, what the family could

not refuse, and what you would write if a complaint were made. 7. Record: the risk screen result, the intervention, the decisions and their reasons; do not record adjectives about people. 8. It cannot substitute for individual treatment where indicated, cannot adjudicate disputed facts, can silence the individual, and can become a totalising theory (any two).

12.12 Reflective task

Write your own confidentiality policy in the three sentences you would actually say aloud, then read it to a peer and ask them to describe, in their own words, what they understood. Most students discover that one of the three sentences is unclear, usually the second, and that the peer heard a promise the therapist had not intended to make. Revise, and bring the final version to the seminar.

A second exercise, done in the same pairs: each student states, in one sentence, the circumstances in which they would break confidentiality without the family's agreement. The sentences are collected anonymously and read aloud. Almost every cohort produces the same two or three situations a child at immediate risk, a serious threat to a partner, a disclosure of abuse by a person who is not in the room and the exercise demonstrates that the ethical position does not depend on the individual therapist's temperament but on a small number of defensible limits that can be stated in advance and held under pressure.

12.13 Readings for this week

- **Your professional code of conduct and ethics**, read in full this week rather than consulted when needed.
- **BACP, AFT or your national association's ethical framework**, with particular attention to the sections on multiple clients, confidentiality and competence.
- **Your jurisdiction's safeguarding guidance**, including the threshold for referral and the definition of harm.
- **Bond (2015)**, Standards and Ethics for Counselling in Action, for the practical ethics of multiperson work.
- **A published professional conduct case** or fitness-to-practise hearing, read for how the standards are applied when things go wrong.
- **Chapter 5 of MFT 704** in this library, on decision models, for the detailed treatment of the affair case.

Next: Week 13 Integration I: three lenses on one family

12.3a Annotated transcript: setting the frame in ninety seconds

The confidentiality statement is usually delivered badly: too long, too legalistic, or as a form to be signed. Here is the opening of a first session, with annotations.

> **T1 Therapist:** Before we start, I want to tell you how I work with information, because we are three people in this room and there is no single rule. Is that all right?
> **T2 Mother:** Yes.
> **T3 Therapist:** Anything said here, with all of us present, stays with all of us. That is the easy part.
> **T4 Father:** And if she comes alone?
> **T5 Therapist:** If one of you comes alone and tells me something, I will encourage you to tell the others, and I will tell you in advance if I think I have to raise it myself. I will not carry a secret about your family and work as though I did not know it.
> **T6 Father:** What if it is not my business to tell her?
> **T7 Therapist:** Then we talk about what to do with it, and you decide, unless it is about someone's safety. If a child is being hurt, or someone is at risk of serious harm, I am not allowed to keep that, and I will tell you before I do anything.
> **T8 Mother:** And the school?
> **T9 Therapist:** Nothing goes to the school without your agreement, except in the safety situation I have described. If I write a letter, you will see it first.
> **T10 Father:** (nods) That is fair.

Annotations. T1 names the structural fact three people, no single rule and asks permission. The permission is not merely courteous: it establishes that the frame is negotiated rather than imposed, which is the first demonstration of how the therapy will work.

T4 is the question the therapist wants, and it comes because she left a gap rather than filling it. Trainees tend to deliver all three categories in a monologue, which produces silence; a short statement followed by a pause produces the question that matters to the family.

T5 contains the crucial phrase: "I will not carry a secret about your family and work as though I did not know it." This is the sentence that prevents the commonest ethical trap, and it is more honest than the usual formula ("what you tell me alone stays alone"), which is a promise the therapist cannot keep.

T7 states the exception in plain language, with two examples, and adds the procedural guarantee: "I will tell you before I do anything." The guarantee is what converts a threat into a protection, and it is what families remember when something goes wrong.

T9 pre-empts the question every family with a school involvement will ask, and it makes a promise the therapist can keep. Note the specificity: "if I write a letter, you will see it first," not "your information is confidential."

What is missing, and should be added in the written policy. The record: how it is kept, who can see it, how it is shared, and what happens if the family separates or if a member requests access. A ninety-second statement cannot contain everything, which is why the written page exists.

12.4a Suicide risk in a family session

The commonest serious-risk situation in family work is an adolescent's disclosure of suicidal thoughts, and the commonest failure is a therapist who does not ask because asking feels like introducing the idea.

The evidence on asking. Asking directly about suicide does not increase risk; there is consistent evidence that screening and asking are safe and that they identify people who would otherwise be missed. The reluctance is the therapist's, not the family's.

What to do when an adolescent discloses, in order.

One: thank them, and slow down. "Thank you for telling me. That took courage. I want to ask you some careful questions, and then I want us to decide together what happens next."

Two: assess. Thoughts, frequency, intent, plan, access to means, previous attempts, what has kept them safe so far, who knows, and what they would want their parents to know. Ask in plain language: "Have you thought about how you would do it?" is a legitimate clinical question.

Three: negotiate disclosure with the young person before speaking to the parents. Not a veto the parents must be told but an agreement about how, when and by whom. "I have to tell your mother. Would you rather do it with me in the room, or would you rather I did it while you wait outside?"

Four: involve the parents as protectors, not as investigators. The conversation with parents should give them three things: what has been disclosed, what the plan is, and what to do tonight. Parents who are frightened become intrusive; parents who are informed and given a task become protective.

Five: means restriction. The single most effective intervention available to a family therapist is the reduction of access to means medication, pesticides, a vehicle, a weapon. Ask: "What is in the house that could be used?" and make a specific plan.

Six: a written safety plan. Warning signs, coping strategies, people to contact, professional contacts, and the means restriction agreed. Written with the young person, in their words, and copied to the parents with their agreement.

Seven: escalate if needed, and record. Same-day psychiatric assessment where intent or plan is present; the family's general practitioner informed with consent; and the reasoning recorded, including what was decided and why.

> **CAUTION**
> Do not manage serious suicide risk in a family session as though the family's presence were a substitute for a clinical risk assessment. The family is a resource, not a plan.

12.5a Mandated families: the ethics of court-referred work

A significant proportion of family therapy is mandated: by a court, a child protection plan, a probation order or a school's conditional enrolment. Mandated work has its own ethical architecture.

Who is the client? The first question, and it must be answered explicitly and in writing. Where the referrer is a court or a local authority, there are three possible answers: the family is the client and the referrer receives reports (the usual arrangement); the referrer is the client and the family are participants in an assessment; or the therapist has a dual role that must be managed in the open. Each has different rules about confidentiality, and the family must be told which one applies before the first session.

Consent under mandate. Consent is not absent because the family is required to attend; it is thinner and must be worked for. "You did not choose to be here. You can choose what you tell me, and how you use the sessions. I am obliged to report on attendance and on risk; I am not obliged to report everything you say, and I will tell you before I write anything."

Reporting. Say what will be reported, to whom and when, in advance. A report that surprises the family is a breach even when it is lawful. Where the report will be read by a court, write it with the standards of 12.7 and give the family the opportunity to correct factual errors before it is filed.

Therapeutic work under mandate. Mandated families can engage genuinely, particularly when the therapist is clear about the mandate, offers something the family actually wants, and does not pretend that attendance is voluntary. The failure mode is the therapist who either becomes an agent of the court (losing the family) or colludes with the family against the court (losing the mandate and the professional standing).

The ethical floor. The therapist does not write a report that misrepresents the evidence, does not accept a referral whose purpose is to produce a predetermined conclusion, and does not conceal a conflict of interest. Where the instruction is to produce a particular finding, the professional response is to decline the instruction in writing.

12.5b Three worked competence decisions

Case 1 the psychotic adolescent. A family is referred for "communication problems"; the sixteen-year-old son has become withdrawn, is not sleeping, and has begun to say that the neighbours are transmitting his thoughts. Competence question: is this family work? Answer: the presentation requires urgent psychiatric assessment, and family work is a component of the care that follows, not a substitute for it. The professional move: telephone the general practitioner and the local mental health service that day, with the family's knowledge; offer the family sessions alongside the assessment; and record the reasoning.

Case 2 the live custody dispute. Parents of two children, separated, each requesting therapy for the children and each asking the therapist to "note what the other one is like". Competence question: can a therapist hold a clinical role in a contested legal process? Answer: not without clarity. The professional move is to state, in writing, that the

therapist's role is clinical, that no report will be provided for the proceedings without a separate instruction and the agreement of both parties, and that any letter will be sent to both parents. Where the court requires a report, it should be commissioned from an independent expert, not from the treating clinician, and the therapist should say so.

Case 3 the family that wants a diagnosis for the father. An adult family brings a 62-year-old father who has become irritable and forgetful; the family wants the therapist to "confirm it is stress". Competence question: is this a psychological presentation? Answer: the change in an older adult requires a medical assessment for neurological and other causes before any psychological formulation. The professional move: state plainly that a medical assessment is needed, help the family arrange it, and hold the family work until the results are known.

12.6a Five dual-relationship scenarios, decided

The test from 12.6 applied, with reasoning shown, so that students can see how a decision is made rather than only what it is.

Scenario A the family who attend your church. Facts: you will see them weekly in a setting where you are a member rather than a therapist. Risks: the family may feel unable to speak about matters that would become known in the congregation; the therapist's own standing may be affected. Decision: usually workable, with three conditions an explicit conversation about what will be said in public, an agreement that you will not acknowledge the therapy at church unless they do, and a review after four sessions. If the family is prominent in the congregation, or the presentation involves matters the community would find scandalous: refer.

Scenario B your cousin's neighbour, in a small town. Facts: information travels; you may be seen as connected to the family's affairs. Risks: the family may censor what they say; you may hear accounts of yourself through the network. Decision: ask the family directly whether they know you, and record the answer. If the connection is genuine and acknowledged, work with a stricter boundary than usual. If the family did not know and would have chosen otherwise, offer an alternative.

Scenario C a former client's sister, referred for couple work. Facts: you hold confidential information about the sister's family of origin. Risks: inadvertent disclosure; the former client may feel betrayed; the sister may ask you questions you cannot answer. Decision: usually workable, provided you (a) obtain the former client's permission where they are identifiable and the information is relevant, (b) tell the new clients what you cannot discuss, and (c) record the arrangement.

Scenario D you gave advice informally at a family gathering. Facts: a clinical relationship is being proposed after an informal one. Risks: the family will treat you as an adviser with a prior view; their account will be shaped by what you already said; you have already formed a view without a proper assessment. Decision: decline, and explain why in one sentence. Offer a referral to a colleague; the family will usually accept.

Scenario E an invitation to speak at the family's church, or to invest in the family's business. Facts: the relationship is being converted from clinical to public or

commercial. Risks: the family may feel unable to refuse; the therapeutic relationship is contaminated by an audience or a financial interest. Decision: decline both while the therapy is live. After discharge, the public invitation may be acceptable if the family initiates it, if no clinical material is used, and if the offer is recorded; the commercial invitation is never acceptable.

The general rule. The question is never "is this comfortable for me?" but "would this family be able to refuse, and would they be able to speak freely afterwards?" Where the answer to either is no, the boundary is in the wrong place.

12.7a A note that survives scrutiny

The same session, written twice.

The bad note.

> "Seen mother and J (14). Mother very hostile and manipulative, clearly enmeshed with J. J sullen, probably depressed. Father not present, as usual. Discussed boundaries. Mother became tearful and blamed everyone. Will review. Risk none apparent."

What is wrong with it. Four adjectives about a person ("hostile", "manipulative", "enmeshed", "sullen"); a diagnosis made in passing ("probably depressed"); a judgement about an absent person ("as usual"); an intervention described in a word that means nothing without context ("discussed boundaries"); no record of what was actually done or agreed; a risk conclusion without a screen; and a tone that the mother would experience as an attack if she ever read it.

The good note.

> "Session 3, 14 October, 60 minutes. Present: mother, J (14). Father absent mother reports he is working away and has been informed of the appointment by text. Presenting: mother reports two arguments this week, both about J's telephone. J reports that mother 'checks everything'. Risk screen: mother and J both denied thoughts of self-harm or suicide. No disclosure of physical harm in the household. J said 'it is not that bad'. No safeguarding concern identified today; screen to be repeated at review. Work: I asked each of them to describe the last argument in sequence. The sequence each described was: J takes the telephone to her room; mother asks for it; J refuses; mother takes it; J shouts; mother leaves the room; both silent for the evening. Both agreed this was accurate. Intervention: I offered the reframe that both of them are trying to keep the peace, in different ways, and that the telephone has become the place where the argument happens. Mother said 'I am not trying to control her'. I did not argue; I asked what she would want instead. She said she would want J to tell her who she is talking to. J said she would tell her if she did not 'make it a police matter'. Agreed task: J to tell mother who she is messaging, once a day, unprompted; mother to ask once and not to take the telephone for one week. Both agreed to try; J said 'I will try'. Plan: session 4 next week, father invited by telephone call from me with mother's agreement. Risk screen repeated. Supervision: discussed the father's absence and my own irritation with the mother. Signed, time, date."

Why the second note is defensible. It records fact and quotation; it records the risk screen and the reasoning; it records the intervention precisely; it records what was agreed by whom; it records the plan and the supervisory discussion; and it contains not one adjective about a person. It is also more useful: a colleague taking over tomorrow could continue the work from it.

12.8a Referring well: the handover

A referral is a clinical intervention, not an administrative act. Four components.

One: the conversation before the letter. Tell the family what you are recommending, why, and what will happen. Families who are referred without explanation experience it as rejection, and a proportion of them do not attend.

Two: a letter that would help a colleague. The reason for referral; what has been assessed; the formulation in three sentences; what has been tried; the risk status and the safety plan; what the family wants; and what you have told them. Two sides of A4 at most.

Three: the warm handover where possible. A joint meeting, or a telephone call with the receiving clinician present, raises attendance substantially. Time-consuming and worth it.

Four: the boundary of your continuing role. Say what you will do next: remain available for consultation, hold the family's file, close the case. Ambiguity here is the commonest cause of families falling between services.

> **PRACTICAL**
> Write a referral letter for the Adeyemi family to an adult mental health service for the mother. Then have a peer read it as if they were the receiving clinician and say what they would still need to know.

12.10a When a complaint is made

Complaints are inevitable in a professional career and are survivable. Five principles.

Respond promptly and personally. A telephone call within 48 hours prevents most escalations. The complaint is usually about being unheard, and hearing it is half the remedy.

Separate the complaint from your self-image. Being complained about does not make you incompetent, and dismissing the complaint does not make you competent. Read it twice before responding.

Record everything, factually. What was said, when, to whom, and what you did. Do not amend the clinical record in a way that could look like retrospective justification; add a dated addendum instead.

Use the formal process, and get support. Tell your supervisor and your indemnity organisation. Do not handle a serious complaint alone, and do not discuss it with colleagues who are not part of the process.

Learn, and say what you learned. Where the complaint is justified, an apology and a specific change are worth more than a defence. Where it is not, a clear, kind explanation is worth more than silence.

The systemic point. Most complaints in family work arise from the multiperson structure: one member feels the therapist sided with another, or feels that information was shared wrongly. The confidentiality policy of 12.3 and the record discipline of 12.7 are, in practice, the best complaint prevention available.

Week 13 Integration I: three lenses on one family

This week is a rehearsal for MFT 702. You will apply three models to the same case and notice that each produces a different question, a different focus and a different first move. The purpose is not to choose a winner but to become fluent enough to choose deliberately.

13.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0 10	What integration is, and three kinds of it	Define assimilative, eclectic and pluralistic integration	Place themselves on the map
10 25	The case, in full	Read the transcript of the first ten minutes twice	Note what they notice on each reading
25 40	The structural lens	Formulate live, with the case on the board	Add the structural diagram
40 52	The Bowen lens	Formulate live; coach the parents on paper	Identify the triangle and the projection
52 64	The solution-focused lens	Formulate live; find three exceptions	Scale the family's confidence
64 78	The comparison	Three plans side by side	Argue for one, then argue against it
78 90	The switching exercise	Demonstrate one therapist changing lens mid-session	Rehearse a switch

13.2 Three kinds of integration

"Integration" is used loosely in the field. Three distinct activities, each with a different logic.

Assimilative integration. The therapist has one primary model and incorporates techniques or concepts from others without abandoning the core. This is how most experienced clinicians actually work: a structural therapist who uses solution-focused questions in the first session, or a systemic therapist who uses an emotion-focused enactment. The advantage is coherence the therapist knows what they are doing and why. The risk is that borrowed techniques are used without the theory that makes them work.

Eclectic integration. The therapist selects from several models according to the case, with no single organising framework. The advantage is flexibility. The risk, well documented, is incoherence: a session that begins structurally, shifts to narrative questioning, and ends with a behavioural task can leave the family confused about what kind of conversation they are in.

Pluralistic integration. The therapist works with the client's preferences and the evidence, using a meta-framework that explicitly includes multiple methods and negotiates them with the family. The advantage is collaboration and fit; the risk is that the therapist's own expertise dissolves into consumer choice.

Where a trainee should begin. With assimilative integration, because a primary model gives a trainee something to be wrong about. A therapist who has no primary model cannot tell whether a failure belongs to the model, the technique or themselves, and therefore cannot learn from it. The primary model is the instrument; the others are the extensions.

The four tests of a defensible integration. Does it have a stated primary frame? Can the therapist say, in any given session, which frame is operating and why? Does each borrowed technique retain the conditions that make it work? Does the family experience one coherent conversation rather than three?

13.2a Your primary model: a diagnostic

Before the seminar, answer six questions about your own practice. The answers are not a test; they are a baseline against which the rest of the programme will make sense.

1. In your first ten minutes with a new family, what do you do? If the answer is "ask each person what they have noticed", you are working structurally. If it is "ask what they have already tried", you are working solution-focused. If it is "ask each person what they feel", you are working emotion-focused. Most trainees have a default, and most have never named it.

2. What do you look at first in a transcript? Alliances, sequences, feelings, language, or the absent member. The thing your eye goes to is your model, whatever you say it is.

3. What do you do when the session gets difficult? This is the most revealing question. A structural therapist reorganises the room; a Bowenian slows down and asks about the process; a solution-focused therapist asks about exceptions; an emotionally focused therapist deepens the affect; a narrative therapist externalises. The default response under pressure is the primary model, and it is the one you need to know.

4. What kind of family makes you anxious? A family that will not speak, a family that shouts, a family that idealises the therapist, a family that asks for a diagnosis. The anxiety marks the limit of the primary frame.

5. What would you have to give up to change models? Coherence, speed, a sense of competence, the approval of a supervisor. Naming the cost is what makes a deliberate change possible, and the cost is usually higher than trainees expect: a therapist who has

spent three years becoming fluent in one frame has invested in that fluency, and the investment is invisible until a family arrives who needs something else.

6. Which of the five lenses in this week do you use least? That is the lens to practise in MFT 702, and the one you are most likely to need with the families who currently defeat you.

Why this matters now. A trainee who cannot name a primary model cannot integrate, because there is nothing to integrate into. The naming comes before the borrowing.

13.3 The case: the Adeyemis, session one

The material below is the data for the whole week. Read it twice: once for the content, once for what you notice.

Referral. "Amara Adeyemi, 16, out of school for four months. School has made a formal referral to the education welfare service. Mother reports she 'cannot manage her'. Father works in Lagos, attends when he can. Younger brother 11, no difficulties reported."

Who attends. Mother (41, nurse, works shifts), Amara (16, Year 11, wants to be a pharmacist), brother (11, brought because there was nobody at home). Father absent "he cannot take time off in the week".

What the mother says. Amara refuses to go to school. She has tried everything: talking, shouting, removing the telephone, prayer, fasting, a conversation with the school counsellor. Amara stays in her room. The mother telephones the school each morning with an explanation. She is tired, and she is frightened of the education welfare officer. She mentions, in passing, that her husband "does not understand" and that when he is home at weekends he tells her she is too soft.

What Amara says. She is not going back. The school is "full of people who think they know you". She has one friend who left in the summer. She says her mother "talks at me". She says her father "does not listen, he just decides things". She denies being bullied, denies self-harm, and says she is "fine" when she is in her room.

What the brother says. "It is quiet when she is at home. When she is at school it is not quiet." Asked what he means, he says: "Mummy is always on the phone to the school."

What is not said. Nobody mentions the father's redundancy in 2019, the two years he was at home, or the paternal grandmother's death in 2022. Nobody mentions the mother's own father, who died when she was 19.

13.4 The structural lens

The formulation. The household has a parental subsystem that is functionally single the mother manages the children alone for five days a week and a cross-generational coalition in which Amara and her mother are locked in a struggle that has the intensity of a marital conflict. The father, when present, enters as a third authority whose interventions are not coordinated with the mother's. The sibling is peripheral and articulate about the family's emotional temperature. The boundary around the parental subsystem is weak in the week and rigid at the weekend.

The first move. Strengthen the parental subsystem before addressing the school refusal: a session with the two parents only (by telephone if necessary), in which they agree a single position on the school, including what will happen on Monday morning, and who speaks to the school. In the joint session, block the mother daughter escalation by moving the conversation to the parents and asking Amara to observe rather than participate for ten minutes a structural move that puts the adults in charge of the decision.

A sample intervention, scripted. "I would like to talk to your parents for a few minutes about Monday, and Amara, I would like you to listen rather than answer, because the decision is theirs to make. Is that all right? Mrs Adeyemi, Mr Adeyemi, by Sunday evening I want the two of you to have agreed one sentence about Monday morning that you will both say, in the same words. Not two sentences, not one each. What would that sentence be?"

What this lens sees. Hierarchy, alliances, boundaries, the sibling's position, and the immediate, practical leverage in the parental subsystem.

What it risks missing. The mother's grief and exhaustion; the meaning of school for Amara; the possibility that the refusal is protective of something the family has not named; and the institutional loop with the school, which a purely structural frame tends to treat as background.

What would falsify it. If the parents agree a single position and the mornings do not change at all within two weeks, the coalition is not the maintaining mechanism, and the formulation needs the next lens.

13.5 The Bowen lens

The formulation. The family's anxiety is high and is being managed by distance (the father's work), by conflict (the mother daughter struggle) and by projection onto the adolescent, who is the family's identified symptomatic member. The mother's own history the eldest of five, her father's death at 19, her care of four siblings while her mother was unwell has produced a pattern of over-functioning under stress and a low tolerance for needing help. The father's history his mother's death when he was 16, his departure for Lagos that year has produced a pattern of managing anxiety by leaving and by being needed at a distance. The marital dyad is emotionally distant, and the distance is stabilised by the focus on Amara. Amara is triangled into her parents' unresolved business, and her refusal is simultaneously a protest and a stabiliser.

The first move. Lower the family's reactivity and increase the parents' differentiation: coach each parent, separately at first, to observe their own part in the cycle without blame, and to manage their own anxiety rather than attempting to manage Amara's. Concretely: a "detriangulation" task in which the mother, when Amara refuses, does not argue, does not telephone the school, and instead says one sentence and leaves the room reducing the emotional intensity of the triangle rather than winning the contest.

A sample intervention, scripted. "When Amara says she is not going, and you feel the anger rising, I want you to notice it and to say one sentence something like 'I am not going to argue this morning' and then to leave the room. Not because it will get her to

school. It will not, at first. Because it takes you out of the triangle and leaves her with her own decision for twenty minutes. Can you do that and tell me what happens?"

What this lens sees. Multigenerational patterns, the transmission of anxiety, triangles, the function of the symptom in managing marital distance, and the therapist's own reactivity as a source of information.

What it risks missing. The school's timetable, which will not wait for differentiation; the practical conditions of the family's week; and the possibility that the problem is better addressed by a direct, pragmatic move than by a slow process of coaching.

What would falsify it. If the mother can lower her reactivity for two weeks and nothing changes in Amara's behaviour or the family's emotional temperature, the triangle is not the central mechanism.

13.5a The same case at session eight: what each lens would call progress

A useful exercise, because it distinguishes the lenses more sharply than their first moves do. The Adeyemis have now had eight sessions. What would each lens regard as the evidence that the work is succeeding?

The structural criterion. The parents have made three decisions together without Amara in the room, and the same sentence has been used in the kitchen on consecutive mornings. The sibling has been asked, once, how things are at home, and has given an answer that was not about Amara. The hierarchy is functioning: decisions are made in the parental subsystem and communicated downwards, rather than negotiated in the triangle.

The Bowen criterion. The mother has described her own reaction in a session without blaming her daughter, and has reported one occasion on which she left the room instead of escalating. The father has described his own experience of being excluded, rather than criticising his wife. The parents have had one conversation about the family's two deaths. The family's reactivity is lower, and it is lower across several domains, not only around school.

The solution-focused criterion. The family can describe, specifically, what a good morning looks like, and can report three occasions on which it happened. The mother's confidence on a nought-to-ten scale has moved from two to four. Amara has described one thing she wants that is not "to be left alone".

The emotion-focused criterion. The mother has expressed the fear and the shame in Amara's presence, and Amara has responded with something other than withdrawal. The father has said what it is like to come home, and has been heard. The cycle has been slowed enough that the family can describe it together.

The narrative criterion. The family has a name for the problem that is not Amara's name. Each member can describe what the problem has cost them. Somebody has produced a unique outcome a morning that contradicts the story and the family can say what it means.

What the exercise reveals. The five criteria are not interchangeable, and a family can meet one and fail the others. A family that has made decisions together but in which

nobody has said anything true about the deaths has met the structural criterion and not the Bowenian one; a family that can describe a good morning but cannot describe what the problem has cost them has met the solution-focused criterion and not the narrative one.

The practical consequence. The therapist should decide, at assessment, which criterion will be the primary measure of success for this family, and say so. "For the next eight weeks, I will judge this work by whether the two of you can make one decision together and hold it. We will look at everything else afterwards." That statement is what a formulation is for, and it prevents the demoralisation that comes from judging progress by every criterion at once.

13.6 The solution-focused lens

The formulation. The family is organised around a problem story that has become total: "Amara refuses school, we cannot manage her, the school is threatening us." Within that story there are exceptions, competences and preferences that nobody has described. The task is not to explain the problem but to build a description of the solution the family wants, and to notice the occasions when it already happens.

The first move. Establish what would be different, in observable terms, and find the exceptions. Questions, in order:

> "Suppose tonight, while you are asleep, the problem is solved. What is the first thing you notice tomorrow morning that tells you something is different?" "What would I see you doing in that kitchen that I would not see now?" "When was the last time it was a little bit like that even for half a morning?" "What did you do that made that possible?" "On a scale of nought to ten, where are you now, and where would one point higher be?"

A sample intervention, scripted. "You said the last good morning was in September, the week of your sister's wedding, when Amara got up without being called twice. What was different about that morning? You had cooked something she likes. What would it take to do that on a Monday, not because it will get her to school, but because it is the kind of morning you want more of?"

What this lens sees. The family's own goals, their existing competences, the small differences that make a difference, and the speed at which a family can move when the conversation is about what they want rather than what is wrong.

What it risks missing. The function of the symptom (what it is doing for the family system); the loss and history that may be driving it; the institutional realities; and the possibility that optimism becomes an implicit demand that the family perform improvement for the therapist's benefit.

What would falsify it. If a well-specified preferred future and two weeks of exception-building produce no change in the mornings and no increase in the family's own sense of agency, the problem is not a solution-description problem.

13.7 Two further lenses, briefly

MFT 702 treats each model in depth. Two more are presented here because comparing three lenses is instructive and comparing five is educational.

Emotion-focused lens. The mother's exhaustion is secondary to a primary emotion the fear of being alone with a child she cannot reach, and the shame of failing at the one thing she believes a mother should manage. The father's weekend interventions are secondary to his own shame at having been absent and his fear of being irrelevant. Amara's refusal is a protest against a bond that has become functional rather than affectionate. The cycle is: mother pursues, Amara distances, mother escalates and then collapses, Amara feels guilty and withdraws further, father enters at the weekend with a directive that confirms both his irrelevance and the mother's resentment. The first move is to slow the cycle down and access the primary emotions, beginning with the mother's, in an enactment in which Amara is asked to listen.

Narrative lens. The family is living inside a dominant story "Amara is the difficult one" that has recruited the school, the mother and, in a different way, Amara herself. The task is to externalise the story ("the Not Going to School problem"), map its influence on each member, and find unique outcomes where the family has resisted it. The first move is to separate the person from the problem and to ask each member what the problem has cost them and what it has taken from them.

Why all five lenses produce a plausible plan. Because the case genuinely contains hierarchy problems, triangles, exceptions, unexpressed emotion and a dominant story. A family is not one thing; it is a system rich enough to support several accurate descriptions. The professional question is therefore not "which description is true?" but "which description gives this family, at this moment, the best chance of change, and what will tell me if I am wrong?"

13.8 The three plans side by side

The existing summary of the three lenses, kept because it is the clearest one-page statement of the comparison.

Model	Key question	First move with the Adeyemis	Likely risk
Structural	What is the hierarchy, and where are the boundaries?	Strengthen the parental subsystem; block the mother daughter coalition	Imposing a structure the family experiences as foreign
Bowen/multigenerational	How does anxiety travel, and who absorbs it?	Coach the parents to manage their own reactivity; map the projection onto Amara	Slow; family may want symptom relief now
Solution-focused	What is already working, and what would "one point better" look like?	Find exceptions to school refusal; scale the parents' confidence	May bypass the function of the symptom

Adding the two further lenses.

Model	Key question	First move	Likely risk
Emotion-focused	What primary emotion is the argument protecting?	Slow the cycle; access the mother's fear and shame in an enactment	Intensity too early; the family may not have the capacity to tolerate it yet
Narrative	What story has recruited this family, and who is it serving?	Externalise the problem; map its influence on each member	May remain at the level of description and never reach the mornings

The comparison in four sentences. Each lens selects a different unit (structure, triangle, exception, emotion, story). Each generates a different first move and therefore a different first session. Each has a characteristic blind spot that the others cover. Each is testable, and the marker of a competent clinician is that they can state what would show their chosen lens to be wrong.

13.8a When the lenses disagree: a decision protocol

A procedure for the moment in the seminar or the clinic when two competent clinicians, using different lenses, produce incompatible plans.

Step 1 establish that the disagreement is real. Often it is not. "You want to strengthen the parents; I want to lower the mother's reactivity" may be the same first move in different language. Ask each clinician to script the first sentence they would actually say. If the sentences are compatible, the disagreement is theoretical.

Step 2 identify the level of disagreement. There are four possible levels, and they require different responses.

Level 1: language. The same move, described differently. Response: agree the scripted sentence and proceed.

Level 2: sequence. Both moves are needed; the clinicians disagree about which comes first. Response: decide by the family's circumstances what will be lost if we delay? and record the decision and the review point.

Level 3: mechanism. The clinicians disagree about what maintains the problem. Response: design an observation that discriminates between them, usually a two-week task with a specific expected result, and agree in advance what each outcome would mean.

Level 4: values. The clinicians disagree about what the family should want, or about what counts as a good outcome for example, whether a daughter should be supported to leave her family's expectations. Response: name the value disagreement explicitly, take it to supervision, and where it cannot be resolved, refer to the clinician whose values match the family's needs, saying why.

Step 3 choose and record. Whatever the level, the decision must be made, recorded and given a review point. The commonest failure in integrated teams is an unresolved disagreement that produces a plan with two incompatible parts, which the family experiences as confusion and which nobody reviews.

Step 4 tell the family what was decided and why. Not the professional disagreement in detail, but the reasoning in plain terms: "We have decided to start with

the two of you, because the mornings are going to be decided by you before they are decided by Amara. In a month we will look at whether that was the right place to start."

The discipline this teaches. Disagreement between models is not an embarrassment to be smoothed over; it is the mechanism by which a team arrives at a better plan, provided it is made explicit and resolved rather than averaged.

13.9 Annotated transcript: switching lens mid-session

A demonstration of a therapist changing frame deliberately, in session three with the Adeyemis. The structural move fails, and the therapist does not repeat it.

> **T1 Therapist:** I would like the two of you to agree one sentence about Monday, in the same words. Mrs Adeyemi, what would the sentence be?
> **T2 Mother:** "You are going to school on Monday." That is what I would say.
> **T3 Therapist:** And Mr Adeyemi the same sentence?
> **T4 Father:** (pause) I would say, "Your mother and I have decided, and we will talk about it on Sunday evening."
> **T5 Therapist:** Those are different sentences. They are both firm. One is a command and one is an invitation to negotiate.
> **T6 Mother:** (to father) You always do this. You come home and you make me the bad one.
> **T7 Therapist:** (pause) Let me stop and say what I am seeing. I asked for one sentence, and within a minute we have the argument that I think happens every weekend. Amara, is this what it is like?
> **T8 Amara:** (nods) Every Saturday.
> **T9 Therapist:** Then I want to change what we are doing. I am going to stop asking you to agree a sentence, because agreeing a sentence is not the problem. The problem is that on Saturday you two have a conversation that is really about the last five days, and Amara is in the room for it. So let us do something different. Mrs Adeyemi, when you heard your husband's sentence, what did you feel?
> **T10 Mother:** (crying) That I am doing it alone and nobody sees it.
> **T11 Therapist:** And Mr Adeyemi, when she said "you make me the bad one"?
> **T12 Father:** (long pause) That I am not in my own house.

Annotations. T5 is a structural observation, made neutrally, and it fails: the parents do not hear the distinction between a command and an invitation, they hear an accusation. A therapist committed to the structural frame would repeat the task, perhaps more firmly, and would spend the session managing a conflict that the task has produced.

T7 is the switch, and it is announced. "Let me stop and say what I am seeing" is the sentence that makes the change of frame explicit rather than a loss of nerve. It also recruits Amara as a witness, which serves two purposes: it gives the therapist information and it interrupts the parents' dyad.

T9 moves from structure to emotion, and the move is justified in the therapist's own words: "agreeing a sentence is not the problem." The therapist has used the failure of the structural task as evidence about the mechanism, which is exactly the discipline described in Week 9's testability clause.

T10 and T12 are the payoff. In four turns the session has moved from a disagreement about a sentence to two statements of primary experience being alone, not being in one's own house that no amount of structural instruction could have produced. The therapist will return to the sentence next week; it will be easier to agree.

What makes this integration rather than incoherence. The therapist has a primary frame (structural), states the observation from that frame, and then makes a deliberate, announced shift to a second frame (emotion-focused) with a reason. She has not randomly changed technique; she has followed the evidence that the first frame's first move was not working.

What would have made it incoherent. Switching without saying so; switching because the therapist was uncomfortable with conflict rather than because of evidence; and switching back and forth three times in a session, leaving the family unable to tell what kind of conversation they were in.

13.9a Rehearsing the switch: three scripts

Frame shifts are a skill and are learned by rehearsal. Three scripts to practise in pairs, each with a distinct justification.

Script 1 from structure to emotion, when the structural task produces conflict.

> "I asked the two of you to agree a sentence, and instead we had the argument. That is useful information, so let me stop the task. Before we go back to the sentence, I want to know what happened inside each of you when it started. Mrs Adeyemi, what did you feel when he gave his version?"

Script 2 from exploration to structure, when the conversation becomes circular and nothing is decided.

> "We have been going round this for twenty minutes, and I notice that each of you has said the same thing three times. I want to change what we are doing. I am going to ask you, as the two adults, to make one decision in this room, now, about Saturday, and I will help you make it. Is that all right?"

Script 3 from problem talk to exception talk, when the family is demoralised.

> "I want to stop for a moment, because everything you have told me this morning has been about what is not working, and I believe you. I want to ask a different question. In the last month, was there one morning that went better than the others, even slightly? There was. What was different about it?"

What all three have in common. Each begins by naming what has just happened ("instead we had the argument", "we have been going round this", "everything you have told me has been about what is not working"). Each states the change of direction. Each

gives a reason. And each asks permission or offers a choice. Those four elements are what distinguish a frame shift from a loss of nerve.

13.10 Laboratory: three plans, one decision

The assessed practical, run over two hours with the Adeyemi data set.

Stage 1 (40 minutes) three one-page plans. Groups of three. Each student takes one lens structural, Bowen, solution-focused and writes a one-page plan containing: the formulation in four sentences; the first move, scripted in the words they would use; the second move; what they expect to see within two weeks; and what would show the plan to be wrong. No plan may exceed one page; the constraint forces selection.

Stage 2 (30 minutes) presentation and challenge. Each student presents for eight minutes. The other two challenge on three grounds only: does the first move follow from the formulation? could the family refuse it without losing face? and what would tell you it is failing?

Stage 3 (30 minutes) the decision. The group must produce one joint plan, with a stated primary frame, a justification for choosing it over the other two, two elements imported from the other lenses, and a review point at which the choice will be reconsidered. The justification must address the family's circumstances, not only the model's elegance.

Stage 4 (20 minutes) the family version. Translate the joint plan into the first two minutes of the next session, spoken aloud, in plain language, ending with a question the family could answer with "no".

Marking rubric.

Criterion	Weight	Full marks
Formulation in each lens	20%	Four sentences; the lens's own logic; no contamination from other models
First move, scripted	20%	Specific words; follows from the formulation; the family could refuse it
Falsifiability	20%	A two-week expectation and a stated condition of failure
The decision	25%	A primary frame, justified by this family's circumstances; two principled imports; a review point
The family version	15%	Two minutes, plain language, one answerable question

The commonest failure, and why. Groups produce three good plans and then choose the one they find most comfortable, without justifying it by the family's circumstances. The marker's question is always: "why this family, this week?" A competent answer refers to the school's timetable, the father's availability, the mother's exhaustion, or the risk of losing the family if the first move is too slow. A weak answer refers to the group's preference for the model.

13.10a A fourth lens: the institution's view of the same case

An exercise that changes how students see the whole week. The same family, formulated by the school.

The school's formulation, written by the head of year. "Amara Adeyemi has 61 unauthorised absences since September. Her mother telephones with explanations that are not supported by medical evidence. The family has not attended two meetings. Amara is a capable student whose predicted grades would allow her to study pharmacy. The school's concern is that the pattern has become established, that the family is not engaging with the support offered, and that the education welfare process will be triggered if attendance does not improve by half term. The school's view is that Amara is being supported at home to remain absent."

What the school sees that the therapist may not. The arithmetic of the absences; the timetable of consequences; the predicted grades, which mean that the loss is greater than the family has acknowledged; the fact that the mother has missed two meetings, which the school reads as non-cooperation; and the legal position of the school, which requires it to act.

What the school cannot see. That the mother telephones every morning because she is trying to protect her daughter and herself from the school's judgement; that the two missed meetings were on days when she was on a seven-o'clock shift; that the family has never been asked what they think is happening; and that the letters themselves are part of the loop.

The integration. A therapist who formulates only the family will write a letter to the school asking for understanding, which the school will read as special pleading. A therapist who formulates only the institutional process will pressure the family, which will produce compliance without change. The competent formulation holds both: "the school is escalating on a timetable, the mother is concealing on a timetable, and the two timetables are producing the absences."

The first move that follows. A meeting at the school, with the mother and the therapist, at which three things happen: the mother tells the truth about the mornings; the school names its legal position plainly; and a staged return is agreed with a named member of staff and a fortnightly review. The formulation has become a joint plan because it included the institution as a member of the system.

The lesson for integration. Every lens in this week is a lens on a family. The family in front of you is usually also a case in somebody else's system, and that system has a formulation of its own, a timetable, and a legitimate view. The most useful integration a trainee can learn is not between two therapy models; it is between a therapy model and the institution's account, held at the same time, without either being surrendered.

13.11 Six errors of integration

1. **The incoherent session.** Three techniques from three models in forty minutes, with no frame the family can follow. Fix: name the frame at the start, and announce any change of frame aloud.

2. **The technique without its theory.** Using an enactment without the alliance work that makes it safe, or a miracle question without the solution-building conversation that gives it meaning. Fix: learn the conditions as well as the technique.
3. **Model loyalty disguised as integration.** "I am integrative" meaning "I do what I always do and call it systemic." Fix: audit your last ten sessions for what you actually did, not what you think you do.
4. **Choosing by comfort.** Selecting the lens that suits the therapist's temperament rather than the family's circumstances. Fix: the "why this family, this week?" test.
5. **The unresolved disagreement dressed as both-and.** "We will do structural work and also explore the past," with no decision about which comes first. Fix: state the primary frame and the sequence.
6. **Integration as avoidance of the evidence.** Treating integration as a reason not to follow a protocol where a protocol has strong evidence (family-based treatment for adolescent anorexia, for example). Fix: the model with the strongest evidence for this presentation is the default; integration is a deliberate departure, recorded and reviewed, and a departure that is not reviewed is simply a preference wearing a method's clothes.

13.11a A second data set: Folake and Dele

A second case, shorter, for students who finish early or who want to practise the comparison without the Adeyemi material they have already discussed.

Referral. "Couple, married nine years, two children (6 and 3). Wife requests help because 'we argue about everything and I do not know if I love him any more'. Husband attends reluctantly."

Who attends. Folake (36, bank officer), Dele (39, runs a small transport business). Both children at school and nursery.

What Folake says. "He is never here. When he is here he is on his phone. I do everything the school run, the homework, the shopping, his mother's hospital appointments. If I ask for anything he says I am nagging. I am tired of being the only adult in this house."

What Dele says. "I work twelve hours a day so that she can have the life she wants. She does not see it. She criticises the one thing I do my business and then she says I am not affectionate. When I come home I am finished. I sit down and she starts."

What neither says at first. Folake's mother died eight months ago, and Folake has been managing her mother's affairs and her father's care since. Dele's business lost its biggest contract four months ago, and he has been borrowing to pay wages.

The structural reading. The couple subsystem has collapsed into a work-sharing arrangement with no emotional boundary; the children and Folake's father have been recruited into the mother's support network while Dele is outside it. The first move: a protected weekly conversation between the adults, with the children's and father's needs scheduled rather than urgent, and an agreement about the household tasks that both can hold.

The Bowen reading. Both partners manage anxiety by over-functioning in their own domains Folake in care, Dele in work and both experience the other's over-functioning as criticism. Their reactivity is high because both have recently been bereaved or humiliated. The first move: coaching each to observe and lower their own reactivity, and mapping the two family histories of over-functioning.

The solution-focused reading. The couple have several exceptions a weekend away in June, a fortnight when Dele was at home after the contract was lost, the way they handled the children's illness in the spring and a clear preferred future that Folake can describe in detail. The first move: build the description of the preferred future and the exceptions into a small, specific experiment.

The emotion-focused reading. Under Folake's criticism is grief and the fear of being alone with her father's decline; under Dele's withdrawal is shame at the loss of the contract and the fear that he is failing as a provider. The cycle is pursue withdraw, and the losses have intensified it. The first move: slow the cycle, and access the grief and the shame, in that order.

The narrative reading. Two competing stories "the man who provides" and "the woman who holds everything together" have become the only stories available, and each requires the other to be inadequate. The first move: externalise the contest between the two stories, and find the occasions when the couple have lived by a third story.

The exercise. Choose one of the five lenses and write the first two minutes of the session, scripted. Then write the sentence you would say if the family asked, "how long will this take?" The answer should reflect the formulation, not the model's marketing.

13.12 Culture, faith and the choice of lens

Model choice is not culturally neutral, and the three lenses differ in what they assume about families.

The structural lens assumes a hierarchy the therapist may need to change. In families where the hierarchy is strongly normative and religiously grounded, "strengthen the parental subsystem" may mean something different from what the therapist intends: it may mean supporting the father's authority in a way that reduces the mother's. The lens must be used with an explicit question: which hierarchy does this family recognise, and does the mother have a place in it that she considers legitimate?

The Bowen lens assumes that differentiation is a good. Families that value interdependence, duty and the subordination of individual preference to family obligation may experience coaching towards differentiation as an invitation to selfishness. The lens can be used, and it must be translated: "managing your own reaction" can be presented as a duty to the family rather than a right of the individual.

The solution-focused lens assumes that the family can describe a preferred future. Families in acute crisis, or families whose difficulties are primarily material, may find the question abstract. The lens works best after the practical questions have been answered.

Faith and the choice of lens. Where a family's account is spiritual, all three lenses are secondary to the family's own framework, and the therapist's task is to work within it rather than to replace it. A structural intervention framed as "the family's covering" and a solution-focused question framed as "what would give evidence that the prayer is being answered" are both legitimate; a lecture on systems is not.

> **DEBATE**
> "Any model, competently delivered, will help most families; the model is the therapist's preoccupation, not the family's." Argue for and against, using this week's case.

13.13 Evidence note: does integration work?

A short note, because students often assume that integrative practice is obviously superior.

What is known. Comparative trials between bona fide family therapies generally find small or no differences in outcome. There is no substantial evidence that integrative practice outperforms single-model practice; nor is there evidence that it is worse. The evidence on therapist effects (Week 11) suggests that who delivers therapy and how well matters more than how many models they use.

What is plausible. Integration may improve fit the match between what the family needs and what the therapist offers and fit is associated with engagement and retention. Integration may also help the therapist persist when a single-model first move fails, which is the mechanism illustrated in 13.9.

What is risky. Integration without a primary frame is associated with drift: the therapist changes approach whenever the session becomes difficult, which teaches the family that difficulty is to be avoided. This is a well-recognised failure mode in supervision, and it is why the primary-frame rule exists.

The defensible claim. Integration is a professional skill that improves fit and persistence; it is not a treatment with an evidence base of its own. Claims for integrative superiority should be treated with the same scepticism the field applies to claims for model superiority, and the clinician's own outcome data should be the arbiter.

13.13a Two transcripts of the same moment, in two frames

A comparison to end the week, using one thirty-second fragment of a session with the Okonkwos, and two therapists' responses to it.

The fragment. Ebuka (9) says: "I do not want to talk about my grandmother."

Response A, structural. "All right. Then let us talk about the mornings. Ebuka, when your mother comes into your room at six, what does she do first?" What this does: respects the refusal, does not pursue the affective material, and returns the session to the behavioural sequence that the formulation identified as the lever. What it costs: the moment of grief is left unaddressed, and Ebuka learns that the therapist will not stay with it.

Response B, emotion-focused. "You do not want to talk about her. That is allowed. Can you tell me what it is like when somebody says her name not about her, about what happens in your body?" What this does: stays with the refusal and makes it speakable; offers Ebuka a way to be in the feeling without being overwhelmed. What it costs: if the alliance is thin or the session is early, Ebuka may shut down, and the therapist will have pushed too soon.

Which is right? It depends on four things, and a trainee should be able to name them: the strength of the alliance; the session number; whether the parent in the room can tolerate the feeling; and whether the family has already had a chance to address the loss. In session two with a mother who has not yet wept, response A is usually correct. In session six, with an alliance and a mother who has described her own grief, response B is usually correct.

The point of the comparison. Neither response is "the integrative one". Integration is not the habit of doing a little of each; it is the capacity to choose, on evidence, and to know what the choice costs. That capacity is developed in supervision, in recorded sessions, and in exercises like this one, and it is the subject of the final week of the course, where the question is not which lens you can use but which kind of clinician you are becoming.

13.14 Key terms

Term	Definition
Assimilative integration	A primary model extended with techniques from others
Eclectic integration	Selection from several models without a single organising frame
Pluralistic practice	A meta-framework that negotiates methods with the client's preferences and the evidence
Primary frame	The model that organises the therapist's formulation and first move
Frame shift	A deliberate, announced change of model within or between sessions
Drift	Changing approach whenever the work becomes difficult, without justification
Fit	The match between what the family needs and what the therapist offers
Lens	A model understood as a way of seeing rather than a set of techniques
Falsification condition	What would show the chosen formulation or plan to be wrong
Sequencing	The decision about which frame or intervention comes first

13.15 Self-test

1. Name the three kinds of integration and one risk of each.
2. Why should a trainee begin with assimilative integration?
3. What are the four tests of a defensible integration?

4. Give the structural first move with the Adeyemis and the condition under which it would be falsified, and say which piece of evidence in the case would most quickly tell you it was the wrong choice.
5. What does the Bowen lens see that the structural lens misses, and vice versa?
6. What is the characteristic blind spot of the solution-focused lens?
7. Why must a frame shift be announced aloud?
8. What does the evidence say about integrative practice, and what is the one defensible claim a clinician can make for it?

Answers. 1. Assimilative (risk: borrowed techniques used without their theory); eclectic (risk: incoherence); pluralistic (risk: the therapist's expertise dissolving into consumer choice). 2. Because a primary model gives the trainee something to be wrong about, and therefore something to learn from. 3. A stated primary frame; the ability to say which frame is operating and why; borrowed techniques retaining their enabling conditions; the family experiencing one coherent conversation. 4. A session with the two parents to agree one sentence about Monday; falsified if the mornings do not change at all within two weeks. 5. Bowen sees multigenerational anxiety and the triangle, and misses the institutional timetable and practical conditions; structural sees hierarchy, alliances and immediate leverage, and misses grief, meaning and the school loop. 6. It may bypass the function of the symptom and the history that sustains it. 7. Because an unannounced shift leaves the family unable to tell what kind of conversation they are in, and it conceals drift from the therapist herself. 8. There is no evidence that integration outperforms single-model practice; it may improve fit and persistence, and it carries a risk of drift.

13.16 Reflective task

Read the Adeyemi data set again, and write down the first three things you noticed. Then compare with what you noticed on the first reading during the lecture. Most students notice different things on the second reading, and the difference is a portrait of their own attentional habits: some notice hierarchy, some notice feeling, some notice what is missing. Write 300 words on what your attention goes to first, and what it costs you.

13.17 Readings for this week

- **Lebow (2014)**, Couple and Family Therapy: An Integrative Map of the Territory, for a systematic comparison of the models on one grid.
- **Nichols & Davis**, the chapters on each of the three models in this week, read side by side rather than sequentially.
- **Minuchin (1974)** on structural technique, and **Bowen (1978)** on differentiation, read for the first moves each author describes.
- **de Shazer (1988)**, Clues: Investigating Solutions in Brief Therapy, for the solution-focused interview in detail.
- **Johnson (2004)**, The Practice of Emotionally Focused Couple Therapy, for the enactment and the cycle.

- **Sprenkle, Davis & Lebow (2009)**, Common Factors in Couple and Family Therapy, for the argument that model allegiance is the least important variable in the room, and for the counter-argument that follows from it.
- **The transcript of your own most recent session**, watched with one lens at a time; there is no better exercise in this week than watching yourself work and asking which model your eye was following, then watching it again with a second lens and counting the moments you missed.

Next: Week 14 Integration II: your emerging clinical stance

Week 14 Integration II: your emerging clinical stance

Write a two-page **position statement**: what you believe about how relational problems develop, what you think therapy does, what you will not do, and what you still do not know. This is not an exam answer; it is the beginning of a professional identity that will be revised for the rest of your career.

14.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0 10	What a stance is, and what it is not	Distinguish stance from model, from opinion, from slogan	Identify stance statements in three quotations
10 25	Five commitments	Theoretical, epistemological, ethical, cultural, acknowledged limit	Draft one sentence for each
25 40	The annotated statement	Read a strong statement and mark its parts	Mark their own draft against it
40 55	Two statements compared	A structural statement and a narrative statement, defended	Argue for the one they do not hold
55 70	The viva	Demonstrate a challenge to a stance, and a response	Take five minutes of challenge each
70 85	The development plan	Supervision, CPD, log, review	Write the first six months
85 90	Endings and continuation	How to leave a course, and a family, well	Plan the next review date

14.2 What a stance is

A clinical stance is the set of commitments that determine what you notice, what you do, what you refuse, and what you are willing to be wrong about. It is not a model, although it includes one. It is not an opinion, because it is testable against practice. It is not a slogan, because it can be stated in sentences that could be contradicted.

What a stance is not, in four common counterfeits.

The badge. "I am a systemic therapist." A model's name is not a stance; it is an affiliation. The stance question is what you believe the model is true about.

The list. "I am integrative and draw on structural, narrative and solution-focused approaches." This is a catalogue, not a position; it does not say what you would do when the approaches disagree.

The aspiration. "I want to be a warm, non-judgemental therapist." A virtue, not a stance; it does not say what you would do with a family that requires a judgement.

The certainty. "I know that diagnosis is harmful." A belief held without a falsification condition, which Week 11 taught you to distrust, whichever direction it points.

What a stance does. It makes your work predictable to yourself and legible to others. It gives a supervisor something to challenge. It gives a family an account of what they are being offered. And it gives you a way of noticing when you have drifted from what you believe.

14.2a Where a stance comes from

A stance is not invented in the final week; it has been forming since the first. Four sources, which students should be able to identify in their own development.

Source 1 the model you were taught best. Most trainees adopt, at least initially, the frame of the teacher who was clearest, or the supervisor who was kindest. This is not a failing; it is how professional identity begins. The question for the final week is whether the adoption was examined or merely absorbed.

Source 2 the families who defeated you. Every trainee has two or three cases that did not work, and those cases shape a stance more powerfully than the successful ones. A therapist whose first family disengaged during a structural enactment may become a therapist who never uses enactment; the mature version of that experience is to learn what the enactment requires and to use it better, not to abandon the technique.

Source 3 your own family. Nobody arrives neutral about hierarchy, loyalty, caregiving, conflict or disclosure. The therapist who was parentified as a child will have a particular sensitivity to parentified children, and a particular risk of over-identifying with them. The genogram of Week 8 and the reflection of Week 10 exist for this reason; the stance should be written with knowledge of the writer.

Source 4 the evidence and the arguments. Week 11 is a source of a stance, not only of information. A therapist who has read the common-factors literature and the specific-factors literature and still believes that the model is the main thing has made a choice; a therapist who has read both and concluded that alliance, fit and technique each matter in different proportions has made a different choice. Both are positions; only one is defended by the evidence as it stands.

The exercise. Identify, in one paragraph each, the teacher, the case, the family pattern and the argument that have most shaped your current position. Most students find that the case they remember is the one that failed, and that the paragraph about their own family is the hardest to write.

14.3 The theoretical commitment

The question. How do you think relational problems develop and change?

What a strong answer contains. A claim about causation (circular, developmental, structural, narrative, or an explicit combination); a claim about what maintains difficulties (the mechanism); a claim about what therapy does (what changes, and how); and an acknowledgement of the model's limits.

Two examples, contrasting.

A structural statement. "I think relational difficulties are maintained by the organisation of the family: by hierarchies that are unclear or inverted, by boundaries that are too diffuse or too rigid, and by cross-generational alliances that recruit children into adult conflicts. Therapy works by changing the organisation by strengthening the parental subsystem, clarifying boundaries and moving decisions to the people who should be making them and by giving the family a different experience of itself in the room. This account is strong where the difficulty is maintained by structure and weak where the difficulty is maintained by meaning, loss or biology, and I use it with those limits in mind."

A narrative statement. "I think difficulties are maintained by the stories that families and institutions tell about their members, and by the practices that those stories make possible. Therapy works by making the stories visible, separating persons from problems, and thickening the accounts of what families value and have already done. This account is strong where the difficulty is maintained by meaning and by the descriptions that others hold of the family, and weak where the difficulty is maintained by material conditions, by acute risk or by biology."

Why the limits must be in the statement. A commitment without limits is an ideology; a commitment with limits is a professional position. The examiner's question will be: "where does this account fail?" and the answer must be in the document.

14.4 The epistemological commitment

The question. How do you know what you know, and how do you know when you are wrong?

Why it is the hardest of the five. Most trainees can state a theory; fewer can state an epistemology, and the epistemological commitment is what distinguishes a clinician from an enthusiast.

Four positions, and how to choose.

Empiricist. Knowledge comes from observation and measurement; the therapist's job is to test hypotheses against data, including outcome data. Strength: accountability, and a defence against conviction. Weakness: the risk of treating what can be measured as what matters.

Constructivist. Knowledge is constructed in language, and there is no view from nowhere; the therapist attends to meanings and to the family's own account, and holds their own account as one account among several. Strength: humility and attentiveness to power. Weakness: the risk of relativism, and of losing the capacity to say that a child is unsafe.

Critical realist. There is a reality independent of our descriptions, and our descriptions are always partial and positioned. The therapist works with hypotheses that are more or less adequate, tests them, and expects to revise. Strength: it holds measurement and meaning together. Weakness: it is harder to state in a sentence.

Pragmatist. What matters is what works, for this family, in this setting; truth is assessed by consequence. Strength: flexibility and a focus on outcomes. Weakness: "what works" for whom, and by whose criteria.

A workable formulation for a first statement. "I work as a critical realist. I assume there is something real happening in this family that my description can be more or less adequate to; I hold my formulation as a hypothesis, test it against what the family says and does and against measured outcome, and I expect to be wrong at least once in every case."

The operational test. Your epistemology is visible in three behaviours: what you do when the family disagrees with your formulation; what you do when the outcome data say you are failing; and whether you can say "I do not know" in a session without filling the silence.

14.5 The ethical commitment

The question. What will you not do, and what will you do even when it costs you the case?

What a strong answer contains. A statement about the family's welfare taking precedence over the therapist's comfort or the model's coherence; a statement about honesty with families, including about the limits of confidentiality and about the therapist's own uncertainty; a statement about competence, including what you will refuse to take on; and a statement about your own accountability supervision, records, complaints.

A worked example. "I will not work with a couple where there is coercion and fear, in joint sessions, whatever the model suggests, because the safety of the person who is afraid takes precedence over the technique. I will tell a family the truth about the limits of confidentiality before I act on them, even when telling them may cost me the work. I will not accept a case beyond my competence in order to avoid appearing inadequate, and I will say so to a referrer in writing. I will keep records that the family could read, and I will take every serious decision to supervision."

Why the negatives matter more than the positives. "I will be warm and respectful" is unfalsifiable and costs nothing. "I will not run joint sessions where there is coercion" is specific, costly, and therefore a real commitment. An ethical statement without a refusal in it is a greeting card.

14.5a A supervision transcript: a stance under pressure

A five-minute excerpt from a supervision session, in which a trainee's stated commitment meets a case that tests it. The trainee has said, in her position statement, that she will not run joint sessions where one partner is afraid.

> **T1 Supervisor:** Tell me about the Ogundipe case.
> **T2 Trainee:** Couple, married fourteen years. She says he has pushed her twice, both times when he had been drinking. He says she "goes on and on" until he loses his temper. They want to stay together. The referrer has asked for couple work.
> **T3 Supervisor:** And what does your stance say about that?
> **T4 Trainee:** That I do not see them together while she is afraid. But the referrer has asked specifically, and there is no other service, and she says she does not want to leave him.
> **T5 Supervisor:** So which of those sentences is your commitment and which is the pressure?
> **T6 Trainee:** (pause) The commitment is the first one. The rest is the pressure.
> **T7 Supervisor:** Good. Now what does your commitment actually leave you able to offer? Not what it forbids.
> **T8 Trainee:** I could see her individually, and him individually, and be clear with both about why. I could write to the referrer explaining the reasoning. I could look for a programme for men who use violence. And I could reassess if the fear went away.
> **T9 Supervisor:** What would have to be true for you to reassess?
> **T10 Trainee:** She would have to tell me, alone, that she is not afraid of him. And I would have to believe her, which is not the same thing.
> **T11 Supervisor:** That last sentence is the most professional thing you have said this morning. Write it down and put it in your statement.

Annotations. T4 is the moment every trainee recognises: the commitment stated in the abstract colliding with a service pressure, a family's preference and a real absence of alternatives. The temptation is to negotiate the commitment away.

T5 is the supervisory question that separates conviction from pressure, and it is worth memorising, because it can be asked of oneself.

T7 is the crucial move, and the one trainees least expect. A commitment that only forbids leaves the therapist with nothing to offer, and the family experiences the ethical position as abandonment. The supervisor's question "what does it leave you able to offer?" converts a prohibition into a service.

T10 is the operational criterion, and T11 is the supervisor's recognition that a stance develops through sentences like this one, not through reading.

The general lesson. A commitment is tested by the cases in which honouring it costs something. The trainee who can name what she will offer instead, and what would change her mind, has a stance. The trainee who can only repeat the rule has a slogan.

14.6 The cultural commitment

The question. How will you work across difference, and what will you do about your own position?

What a strong answer contains. A statement of your own cultural location, honestly described, including the advantages it confers; a statement about the method you will use to learn about families who are not like you (asking, not assuming); a statement about the

power in the room and how you will use it; and a statement about the limits of your competence across difference, with a plan.

A worked example. "I am a middle-class, university-educated woman from a Yoruba family, working in a city where most of the families I see are poorer than I am and many are from other ethnic groups. I will ask about the family's own arrangements rather than infer them, I will not use the family to educate me, and I will notice when I am working harder with a family because of their background which is a form of treating them differently. I will seek supervision from colleagues with knowledge I do not have, and I will record when I have adapted an approach and why."

The trap to avoid. The cultural commitment that consists entirely of good intentions ("I will be culturally sensitive and aware of my biases") is the least useful sentence in the document. The commitment must specify behaviour: what you will ask, what you will not assume, and who you will consult.

14.7 The acknowledged limit

The question. What do you not know, what are you not yet able to do, and how will you know when you have reached the edge of your competence?

Why this section is the one examiners look for. Because a position statement without a limitation is a performance. The limitation section is the place where a trainee demonstrates that they can hold their own practice as an object of scrutiny, which is the whole point of a professional training.

What a strong answer contains. A specific gap (not "I have much to learn"), with its consequences for a family; the mechanism by which you will notice it (supervision, outcome data, a colleague's feedback, a pattern in your caseload); and the plan for closing it.

Four examples of specific limitations.

"I am not yet reliable with high-conflict separated parents. I become anxious when the conflict is in the room and I try to smooth it, which deprives the children of the adult who can hold the difficulty. I will notice it in supervision, where my supervisor has already begun to point it out, and I will take the MFT 719 module and seek co-working on two such cases this year."

"I over-function with mothers. The pattern is visible in my caseload: I give more advice to mothers than to fathers, and my session recordings show me asking mothers three times as many questions. I will count the questions in my next ten recorded sessions and review them with my supervisor."

"I do not work well with families whose first language is not mine without an interpreter, and I have twice proceeded without one in order not to lose the appointment. I will not do that again; I will reschedule, and I will record the reason."

"I am not competent to assess risk of serious violence, and I know that I sometimes reassure myself that a situation is 'probably fine'. My plan is to use the structured screen

every time, to consult the safeguarding lead whenever the screen is positive, and to say in supervision when I have noticed myself wanting to move on."

The test of a limitation statement. Could a colleague observe you and tell whether you had addressed it? If not, it is a confession rather than a plan.

14.8 Writing the statement: structure and a worked model

The structure, in six sections and two pages.

1. **What I believe about how relational problems develop and change** (400 500 words). The theoretical commitment with its limits.
2. **How I know what I know** (250 350 words). The epistemological commitment, with the operational test.
3. **What I will not do** (250 350 words). The ethical commitment, in refusals.
4. **How I will work across difference** (250 350 words). The cultural commitment, in behaviours.
5. **What I do not yet know, and how I will find out** (250 350 words). The limitation with a plan.
6. **What therapy is for** (150 250 words). The sentence that holds the whole document together.

The final section, which students find hardest. "What therapy is for" requires a value claim, not a method claim. Two examples. Example A: "Therapy is for restoring a family's capacity to go on to face what is happening without splitting into blame, and to make the next decision together." Example B: "Therapy is for widening the range of stories a family can live by, so that a member is not reduced to the problem that brought them."

A worked model statement, shortened. The following is a complete statement at about 900 words; the assignment requires two pages, and the model is provided to show the shape.

> What I believe. I believe that most of the difficulties I will be asked to help with are maintained by patterns that no single person intends: cycles of pursuit and withdrawal, coalitions across generations, stories that have hardened into descriptions of a person, and the practical arrangements work, housing, money, migration that make some patterns inevitable. I believe the pattern is the unit of intervention, and the person is the unit of respect. I believe therapy changes patterns by making them visible and by giving the family a different experience of itself, usually before it has a different understanding of itself. I believe this account is strongest where the difficulty is maintained relationally, weaker where it is maintained biologically, and weakest where the family's problem is material and my room is the only place they can talk about it. How I know. I work as a critical realist: I assume something real is happening, that my description of it is partial, and that the family's description is also partial. I test my formulation by its consequences does the family recognise it, does the first move produce the change I expected, do the measures move and I expect to revise it. My operational test of this commitment is what I do when I am wrong in the room, which should be to say so and to ask, not to explain. What I will not do. I will not run joint sessions where one partner is afraid of the other. I will not keep a secret that changes the work. I will not tell a family that their problem is cultural when I have not asked. I will not accept a case beyond my competence in order to avoid seeming inexperienced, and I will say so in writing to the referrer. I will not write a sentence in a record that I would not say to the family's face. How I will work across difference. I will ask about the family's arrangements who decides, who is consulted, who is raising the children, what may not be spoken outside rather than infer them from their background. I will notice when I am working harder with a family because of who they are, and I will take that to supervision. I will not use a family to educate me. What I do not yet know. I am not yet reliable when a family's anger is directed at me. My recordings show me becoming conciliatory and losing the thread of the session. My plan is to review three such recordings with my supervisor this term, to practise staying in the room with the anger in role-play, and to ask for feedback from a co-worker on two live cases.

Why this model works, and what it lacks. It is specific, it contains refusals, it contains a falsification condition, and it contains a limitation with a plan. It lacks the breadth that a full two pages allows in particular, a fuller account of the family's own agency and of the therapist's use of self which is exactly what the assignment requires the student to supply.

14.9 Two statements compared

Two students in the same cohort, both competent, with very different positions. Reading them side by side demonstrates that a stance is a choice, not a template.

Statement A the pragmatic structuralist. "I believe families get stuck in arrangements that nobody chose: a mother managing alone, a father at the edge, a child carrying the worry. I believe the most useful thing I can do is to change the arrangement who sits where, who decides what, who speaks to whom and that insight usually follows change rather than causing it. My evidence is behavioural: does the morning go differently, do the parents agree a sentence and hold it, does the child get to be a child

again. My limits are with families whose difficulty is primarily about meaning or loss, where I tend to move too fast to a task."

Statement B the meaning-oriented narrative therapist. "I believe people are not the problems they arrive with, and that families are often trapped by descriptions that others schools, relatives, clinics have helped to write. I believe the most useful thing I can do is to make those descriptions visible, to separate the person from the problem, and to find the occasions when the family has already lived by something better. My evidence is in what the family can say about itself at the end of the session that it could not say at the beginning. My limits are with acute risk and with families whose difficulties are material, where a conversation about meaning is a luxury and a practical move is required."

What the comparison teaches. Both statements are defensible, both contain limits, and both could be falsified: A's by a family whose morning improves while the meaning does not, B's by a family who can retell their story and still cannot get to school. The examiner's question "which family would you send to the other clinician?" is answerable from the limits section, which is why the limits section is the most useful part of the document.

14.9a The statement as a working document

Students treat the position statement as an assignment to be submitted and forgotten. It is more useful than that.

Use it in supervision. Give it to your supervisor in the first month of placement and ask them to hold you to it: "you said you would not run joint sessions where there is coercion; tell me when you see me drifting." A supervisor who has your statement can challenge you with precision, which is far more useful than generic feedback.

Use it with families. Not the whole document, but its operative sentences. A therapist who can say, in the first session, "I work in a particular way: I will want to hear from everyone, I will be direct about what I see, and I will tell you if I think we are not making progress" is giving the family something to consent to. That is a statement of stance in ninety seconds, and it builds the alliance more effectively than warmth alone.

Use it when choosing a job or a training. A service that cannot accommodate your stated commitments because its model, its waiting list or its management will not tolerate them will require you to compromise them daily. Reading your own statement against a job description is a professional act, not an indulgence.

Use it when you are stuck. When a case is not moving, the statement is a diagnostic instrument. Which of your commitments is the case testing? Which limit have you reached? Which refusal are you being tempted to abandon? Most impasses in clinical work are a stance problem before they are a technique problem.

Use it when you are challenged. The statement gives you something to defend and something to revise. A therapist without one defends their habits; a therapist with one can say "that is a fair challenge to my epistemology, and I will think about it", which is the most professional sentence in the language.

14.10 The viva: defending a stance under challenge

The position statement is examined in a twenty-minute viva. What is assessed is not whether the examiner agrees with you but whether you can hold a position under pressure without rigidity or collapse.

The seven questions examiners ask, and what each is testing.

1. **"Where does your account fail?"** Tests whether you have a limits section you actually believe.
2. **"Give me a family you have seen where your approach would not have worked."** Tests whether the stance is connected to practice, or is an essay.
3. **"What would you do if the family rejected your formulation?"** Tests the epistemological commitment in action.
4. **"You say you will not do X. What if the family asks for X and there is no one else?"** Tests whether the ethical commitment survives pressure; the correct answer names what you would do instead, and what you would refuse.
5. **"How do you know you are helping?"** Tests whether measurement is part of your practice or a phrase in the document.
6. **"Who are you in the room, and how does that shape the work?"** Tests self-knowledge and the use of self.
7. **"What will you have changed your mind about in five years?"** Tests whether the stance is alive.

An annotated answer to question 4. Challenged: "You say you will not run joint sessions where there is coercion. This couple has been referred by a court and there is no other service. What do you do?"

> "I do not run joint sessions where one partner is afraid, because the risk of an escalation after the session is greater than the benefit of the joint work, and because a person who is afraid cannot speak freely in the same room. What I would offer instead is individual work with each partner, a group programme for the person using violence where one exists, a written report to the court explaining what I am offering and why, and a clear statement to both partners about what I will and will not do. If the court requires joint work as a condition, I would explain in writing why I am not the right clinician for it."

Why this answer is strong. It names the refusal, gives the reason, offers alternatives, and takes the consequence. It does not waver and it does not moralise.

The three failures in a viva.

Rigidity. Answering every challenge with a restatement of the position, which suggests the position has never been tested.

Collapse. Abandoning the position at the first challenge ("you are right, perhaps I should be more flexible"), which suggests the position was borrowed for the occasion.

Deflection. Answering a question about limits with a statement of strengths, which is the commonest and the easiest to spot.

14.10a A worked viva, in full

The examination, condensed to eight exchanges, with the board's assessment at the end.

> **EXAMINER:**
> Your statement says you work structurally and that your model fails where meaning is the problem. Give me a family where it failed. **Candidate:** A couple where the wife had a stillbirth two years ago and both had stopped speaking about it. I reorganised the household tasks, which worked, and the silence remained, and I did not know how to enter it. I kept doing structural work because it was producing change, and I mistook change for progress. **Examiner:** What would you do now? **Candidate:** I would name the silence in the first or second session and ask what the couple had been told about talking about it. I would use the enactment to slow the moment down rather than to solve the tasks. **Examiner:** You say you will not keep a secret that changes the work. A mother tells you her son is using drugs and asks you not to tell the father, whom she says will beat him. What do you do? **Candidate:** I tell her, in that session, that I cannot hold it, and why; I ask her consent for a joint session in which she tells him with me present; I check the risk to the son before that session; and if she refuses, I say what I will do next and by when. I would also consider whether the father's response constitutes a safeguarding risk in itself, which it may. **Examiner:** How do you know you are helping? **Candidate:** Session-by-session measures from each member, a goal set in the family's words, and a review point at which I say out loud whether we are on track. And my supervisor sees one recording a month. **Examiner:** What will you have changed your mind about in five years? **Candidate:** I expect to have changed my mind about how much I use enactment early in the work. My recordings show me using it too soon with families who are not yet safe enough with each other, and I expect that to be the thing I look back on as naive.

The board's assessment. Three criteria, typically. Coherence: do the answers follow from the statement? Specificity: are there cases, words and dates, or only abstractions? Openness: does the candidate hold the position without defending it against evidence?

The candidate's strongest move, and its lesson. In answer two, she said "I mistook change for progress". That sentence demonstrates that the failure was examined rather than survived, and it is the kind of sentence that distinguishes a reflective practitioner from a competent technician. The lesson for the reader is not to memorise it but to find the equivalent sentence in your own practice before the viva, because the examiners will look for it.

14.11 The development plan: the first six months

A stance without a plan is a wish. The final section of the assignment is a development plan with four elements.

Supervision. Who, how often, what for, and what you will bring. A plan that says "I will attend supervision" is not a plan. A plan that says "I will bring one recorded session each

month, chosen for difficulty rather than success, and I will ask my supervisor to challenge my over-functioning with mothers" is.

Continuing professional development. Three specific activities with dates: a course, a reading programme, a conference. Each should address the limitation identified in 14.7, not merely the therapist's interests.

Outcome measurement. What you will measure, with what instrument, at what interval, and what you will do with the data. A clinician who commits to measuring outcomes in the first six months will still be measuring in five years; one who intends to begin later usually does not.

The record of practice. A log of sessions with the eleven fields listed in the course requirements. The log is not an administrative burden; it is the instrument of reflective practice, and the students who keep it well are, in the experience of most programmes, the students who develop fastest.

The fourth element that students forget: rest and support. The first six months of practice involve a volume of emotional exposure that most trainees underestimate. A plan that does not include protected time, peer support, personal therapy where appropriate, and a limit on the number of complex cases carried simultaneously is a plan for burnout. This is not a soft addition; it is a competence, and it belongs in the document. The clinicians who leave the profession in their first five years rarely do so because they lacked technique; they leave because they had no plan for the emotional load, no peer group, and no permission to be finite, which is a lesson the profession teaches too late and too often by losing people it could have kept.

14.11a Where this course leads

The final act of integration is to see the programme as a whole, and MFT 701 as its foundation.

The sequence. MFT 702 takes the models introduced in Week 13 and works through each in depth, with its own formulation, techniques and evidence. MFT 703 706 build the practical competences assessment, working with couples, with children, with the wider system. MFT 707 708 address the contexts in which families live: health, schools, faith, and the legal system. The second semester's modules (709 715) develop specialisms and research skills; the third (716 721) returns to complex presentations and to integration at an advanced level. The thesis (770) is where a student's own stance is tested against data.

What MFT 701 supplies to each. To 702: the vocabulary and the capacity to compare models. To 703 706: the first session architecture, the negotiated definition, the genogram and the formulation template. To 707 708: the cultural and ethical frame, and the discipline of writing for institutions. To 709 715: the evidence literacy of Week 11 and the habit of measurement. To 716 721: the ability to say what the systemic frame can and cannot do. To 770: the capacity to hold a hypothesis and to test it.

What the student must supply. Three habits, which no module can teach by itself: the habit of recording sessions and reviewing them; the habit of measuring outcomes and reading the data honestly; and the habit of taking the difficult case to supervision rather

than carrying it alone. A graduate with these three habits will outgrow the curriculum. A graduate without them will have learned a vocabulary.

14.12 Ending well: how the course, the placement and the therapy end

The final clinical skill of the course is the one least taught: how to end.

Ending with a family. Endings should be planned, named in advance, and reviewed. Four elements.

Name it early. "We said we would review in eight sessions; this is the eighth, so today we will look at what has changed and decide together what happens next."

Review the work, with the family's own data. The measures, the goals of Week 7, the family's own statements. Ask what helped, what did not, and what they would say to another family in the same position.

Address the ending itself. Families often have strong feelings about ending, especially families for whom endings have been losses. Ask directly: "What is it like to be finishing?" and be prepared for anger, relief, or a new problem that appears in the penultimate session.

Plan the future. What the family will do if the difficulty returns; who to contact; whether a review session in three months would be useful; what the general practitioner will be told.

Ending badly, and the four common forms. Drift, where sessions become less frequent and stop without a conversation; the unplanned termination, where a family stops attending and the therapist does not follow up; the ending in crisis, where a new problem is produced to avoid the ending; and the ending in disappointment, where the therapist communicates that the family failed to use the work.

Ending the course. The same principles apply. Name the ending; review what has been learned; acknowledge what has not; and plan the next stage. A training that ends in exhaustion and relief is a training that has not taught endings. The final week is designed as a review, not a race.

Ending with a supervisor. Supervision ends too, and the last session should include an honest account, from both directions, of what the supervisee has and has not developed. The habit of asking a supervisor "what would you say I still cannot do?" is one to carry through a career.

14.12a The last session with a family, annotated

An ending, as it should be done, in eight turns.

> **T1 Therapist:** We agreed eight sessions, and this is the eighth, so today I want to look at what has changed, what has not, and what happens next. Is that all right with everyone?
> **T2 Mother:** It is. I am nervous.
> **T3 Therapist:** Tell me about the nervousness.
> **T4 Mother:** I am afraid we will go back to how it was.
> **T5 Therapist:** That is a reasonable fear. Let us look at the evidence, because you have some now. Your ratings went from 28 to 17. Amara has been in school for six weeks. The two of you have made three decisions without a shouting match. What would you say made the difference?
> **T6 Father:** We stopped waiting for her to change first.
> **T7 Therapist:** Amara, what would you say?
> **T8 Amara:** (pause) My mum stopped shouting in the morning. So I stopped pretending to be asleep.

[Twelve minutes later, after the family's review.]

> **T9 Therapist:** Before we finish, I want to ask what it is like to be finishing, for each of you.
> **T10 Mother:** I do not want to stop.
> **T11 Therapist:** That is worth saying, and worth taking seriously. We agreed a review in three months; if something happens before then, you can telephone me, and here is what to do if it is urgent. What would you want me to know if you came back?
> **T12 Amara:** That it is not only me.

Annotations. T1 names the ending at the start of the session, which gives the family the whole session to prepare. T5 reviews the work with data and then asks the family to explain the change, which is the single most powerful thing an ending can do: it leaves the family with its own theory of its own improvement.

T8 is the sentence the therapist will remember, and it is also clinical information: the change was systemic and bidirectional, and the family has described it.

T9 addresses the ending directly. The therapist does not reassure the mother at T10; she takes the feeling seriously and converts it into a plan. T11 names the revisit route and the emergency route separately, which is what a family needs in order to end safely.

What the therapist did not do. She did not add a ninth session because the family was anxious; she did not offer a new goal to soften the ending; and she did not tell the family that they would be fine. Endings are managed by naming them, reviewing them and planning after them, not by avoiding them.

14.13 Six errors in developing a stance

1. **The borrowed stance.** Adopting the position of a charismatic trainer without testing it against your own case material. Fix: write the statement from your own sessions, with examples.
2. **The statement without refusals.** Ethics as aspiration, with nothing you would decline to do. Fix: at least three specific refusals, each with a reason.

3. **The unexamined epistemology.** A theory with no account of how you know when you are wrong. Fix: state the operational test what you do when the family disagrees, when the data fail, and when you do not know.
4. **The universal claim.** A model presented as adequate for everything. Fix: the limits section, with a named presentation where the model fails.
5. **The performance of humility.** A limitation section that is really a strength ("I care too much"), which no colleague could observe and no family could be protected from. Fix: a limitation a supervisor has already named, with a plan and a date.
6. **The stance that never changes.** A statement written once and never revisited. Fix: an annual review, dated, with what changed and why.

14.14 Key terms

Term	Definition
Clinical stance	The set of commitments that determine what a therapist notices, does, refuses and doubts
Theoretical commitment	A claim about how difficulties develop and change, with its limits
Epistemological commitment	An account of how you know, and how you know when you are wrong
Ethical commitment	What you will not do, and what you will do at cost
Cultural commitment	How you will work across difference, specified in behaviours
Critical realism	The position that there is a reality our descriptions are more or less adequate to
Falsification condition	What would show a belief or plan to be wrong
Development plan	Supervisory, educational, measurement and support commitments with dates
Planned ending	An ending named in advance and reviewed with the family
Reflective practice	The disciplined examination of one's own work, usually with records and supervision

14.14a The three-year review: keeping a stance alive

A stance is revised, and the revision is the evidence that it was real. A structure for the review, to be done annually or at the end of each year of practice.

Review 1 the case that changed your mind. Name one case from the year in which you were wrong about something important: the mechanism, the family's capacity, your own reaction, the pace. Write 300 words on what you now believe differently. If there is no such case, the year has not been examined.

Review 2 the pattern in your caseload. What kinds of families do you see most, and what kinds do you avoid or lose early? A caseload is a portrait of a therapist's limits. Compare your outcomes for the families you find easy and the families you find difficult; the difference is your development agenda.

Review 3 the outcomes, honestly. What proportion of your families improved, stayed the same, and deteriorated? Most clinicians never answer this question, and the ones who do become better clinicians because of it.

Review 4 the statement itself. Read last year's position statement. Which sentences still hold? Which are now embarrassing? Which commitment did you honour, and which did you quietly stop honouring? Rewrite the section that has changed and date it.

Review 5 the support. Who knows your work well enough to challenge you? If the answer is nobody, that is the most important finding of the review, and the first item in next year's plan.

Why the review is part of this week's work. Because a position statement that is never revised becomes a description of a therapist who no longer exists, and because the habits established in training are the habits of a career. The students who keep the review are, in most programmes, the ones who are still learning at fifty.

14.14b Two endings, and what each teaches

Ending A the family that left early. A family attended three sessions and then stopped. The therapist wrote a letter offering a further appointment, telephoned once, and recorded: "Family did not return; letter sent; telephone not answered; case closed with the door open. What I would do differently: the father's work pattern was mentioned in session one and I did not ask about it until session three, by which time the mother had decided I did not understand the household. I will ask about the week in the first session, every time."

What this ending teaches. That an ending is data. The therapist has not blamed the family and has not blamed herself; she has identified one specific procedural error and made one specific commitment. That is what reflective practice looks like when it is not a ritual.

Ending B the family that stayed too long. A family attended twenty-eight sessions. The work was going well, the family was engaged, and the therapist did not raise the question of ending until the family's attendance became intermittent. At the review the mother said: "We kept coming because we thought you would be disappointed if we stopped."

What this ending teaches. That endings must be planned from the start, and that a therapist's pleasure in being needed can extend a therapy beyond its usefulness. The professional question at every review is not only "is this helping?" but "is this still the best use of this family's time and this service's capacity?"

The comparison. The two failures are opposite and equally instructive: one ended too soon for reasons the therapist could have anticipated, the other continued too long for reasons the therapist could have named. Both were discovered by review rather than by intuition, and both produced a specific change in practice.

14.15 Self-test

1. What are the five commitments of a position statement?
2. Why must the theoretical commitment include its limits?

3. Give the three behaviours in which an epistemology becomes visible.
4. What distinguishes an ethical commitment from an aspiration?
5. What is the test of a limitation statement?
6. Name the three failures in a viva.
7. What are the four elements of a planned ending with a family?
8. Why does a development plan include rest and support, and what happens to a plan that omits them?

Answers. 1. Theoretical, epistemological, ethical, cultural, and acknowledged limitation (with a statement of what therapy is for). 2. Because a commitment without limits is an ideology, and because the examiner and the family need to know where the model fails. 3. What you do when the family disagrees with your formulation; what you do when outcome data show failure; whether you can say "I do not know" without filling the silence. 4. An ethical commitment contains specific refusals that cost something; an aspiration contains virtues that cost nothing. 5. Whether a colleague could observe you and tell whether you had addressed it. 6. Rigidity, collapse and deflection. 7. Name the ending early; review the work with the family's own data; address the ending itself; plan the future. 8. Because the first six months involve an emotional exposure that trainees underestimate, and a plan that ignores it is a plan for burnout rather than for development.

14.16 Reflective task

Read your Week 1 reflection again. What has changed? What has not? Write 300 words to yourself, dated, to be reread at the end of the programme. Then add one sentence to the end of your position statement that you would not have been able to write in Week 1, and say why you could not have written it then.

14.17 Readings for this week

- **Your own log, notes and Week 1 reflection**, read in sequence, from the beginning of the course to now. This is the primary text for the week.
- **Cecchin (1987)**, "Hypothesizing, circularity and neutrality revisited", Family Process, for the stance of curiosity and its limits.
- **Anderson & Goolishian (1992)**, on the not-knowing position, read alongside Week 12's limits.
- **Schoen (1983)**, The Reflective Practitioner, for the account of professional knowledge in action that underlies the whole idea of a stance.
- **Your professional body's statement of competence**, read as a specification against which to audit your own development plan.
- **Any published first-person account by an experienced family therapist** of how their practice changed over a career, read for the reassurance that a stance is supposed to change, and for the specific sentences in which the author names what they got wrong.

- **Your own recordings from Weeks 1 to 14**, watched in sequence for twenty minutes each; the difference between the first and the last is the most direct evidence you will ever have that the course did something.

Next: MFT 702 Theories of Family Therapy

Integration: the whole course on two pages

1. The frame. Treat the relationship as the client. Widen the field; include context; notice the observer.

2. Causality. Move from who-started-it to what-maintains-it. Draw the loop. Punctuate differently.

3. History. Five movements: pre-history, research rupture, heroic era, critique and meaning, evidence and integration. Each school answers a limitation of its predecessor.

4. Concepts. System, subsystem, boundary, feedback, homeostasis, rule, triangle. Six concepts, most of the clinical work.

5. Profession. Know your scope, your setting, your team, your limits. Refer well.

6. The first session. Join, contract, hear everyone, screen for risk, map the pattern, summarise, invite correction, agree next step.

7. The problem definition. Specific, interactional, owned, changeable.

8. The genogram. Three generations, structural and temporal hypotheses, cultural completeness.

9. Formulation. 5-P plus "who is involved" and "what function the symptom serves", ending with one intervention sentence.

10. Culture and power. Ask, locate yourself, notice power, interrogate normal.

11. Evidence. Family therapy works; alliance matters; models overlap; drop-out is the silent failure.

12. Ethics. Confidentiality is negotiated, risk is screened, competence is bounded, dual roles are examined, notes are written for strangers.

13. Integration. Multiple lenses; move between frames without losing coherence.

14. Stance. A written, revisable position on what you believe and what you do not know.

Glossary

Boundary the implicit rules governing participation and closeness between subsystems. **Circular causality** an account in which each element both causes and is caused by the others. **Coalition** an alliance of two or more people against a third. **Cybernetics** the study of control and communication in systems; first-order (observer outside), second-order (observer inside). **Detriangulation** the process of extricating a third party from a dyadic conflict. **Enmeshment** diffuse boundaries with excessive emotional sharing and low autonomy. **Disengagement** rigid boundaries with little emotional contact. **Feedback (negative)** a process that reduces deviation and restores stability. **Feedback (positive)** a process that amplifies deviation and produces escalation. **Genogram** a three-generation diagram of family structure, relationships and events. **Homeostasis** the system's tendency to maintain its existing organisation. **Hypothesis** a provisional, testable explanation that guides enquiry. **Joining** the therapist's deliberate accommodation to a family's style in order to work with it. **Linear causality** an account in which A causes B in one direction. **Punctuation** the way each participant marks the beginning of a sequence, thereby assigning blame. **Reframe** relabelling a behaviour so that it is understood in a new, usually relational, frame. **Subsystem** a unit within a system organised by boundaries (couple, siblings, parental dyad). **System** a set of interacting elements such that a change in one changes the others. **Triangle** a three-person configuration that absorbs and stabilises dyadic tension.

Self-assessment

Multiple choice (answers below)

1. A therapist says, "The real question is not who started the argument but what keeps it going." This statement reflects: (a) linear causality (b) circular causality (c) psychodynamic causality (d) moral causality

2. A father's drinking has the effect of preventing his wife and son from discussing the son's expulsion. In systemic terms the drinking is: (a) random (b) homeostatic (c) an individual symptom only (d) irrelevant to the family

3. "We never discuss money" is best described as: (a) a boundary (b) a family rule (c) a triangle (d) a reframe

4. In second-order cybernetics the therapist is: (a) outside the system (b) part of the observed system (c) a neutral observer (d) irrelevant

5. The best first move with a family that arrives furious at the referrer is: (a) correct the misunderstanding immediately (b) acknowledge their experience of being sent (c) begin the assessment (d) ask who is the identified patient

6. A negotiated problem definition should be specific, interactional, owned and: (a) clinically imposed (b) changeable (c) blame-allocating (d) historical

7. Which is not an element of a minimum first session? (a) confidentiality and limits (b) a risk screen (c) a full genogram (d) a summary and invitation to correct

8. The strongest predictor of drop-out in couple and family work is: (a) diagnosis (b) therapist's model (c) early engagement and early change (d) family size

9. "Multigenerational transmission" belongs to which tradition? (a) structural (b) Bowen (c) Milan (d) solution-focused

10. The statement "expert in process, not in their lives" describes: (a) neutrality about violence (b) the therapist's stance (c) a legal requirement (d) a diagnostic rule

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 c 8 c 9 b 10 b

Short answer (150 words each)

1. Explain the difference between negative and positive feedback using a family example.
2. Why is "punctuation" clinically useful? Give a sample question.
3. Name three limits of confidentiality in family work and how you would state each to a family.
4. What is the difference between a genogram and a formulation?
5. Summarise the common-factors critique of model loyalty in four sentences.

Essay questions (choose one, 1,500 words)

1. "Family therapy is a frame, not a technique." Discuss with reference to at least two models and one case from your placement.
2. Critically evaluate the claim that systemic neutrality is impossible and undesirable in cases of domestic violence.
3. Using a disguised case, demonstrate how a linear formulation and a circular formulation would lead to different treatment plans. Which would you choose and why?

Practical and clinical log requirements

For each session you observe or conduct, record:

Field	Requirement
Date and duration	Exact
Attendees	Relationship to index client; ages; who was absent
Referral source and question	In the referrer's words
Presenting problem	In each attendee's words
Risk screen	Completed Y/N; result; action
Pattern observed	One loop or sequence, described behaviourally

Field	Requirement
Intervention	What you did, and why
Response	What happened next
Supervision	Who, when, what was discussed
Plan	Next step, with review date

The log is assessed for **specificity and consistency**, not for how impressive the case was. A log of ordinary sessions written precisely will score higher than a dramatic case written vaguely.

Further reading

Core texts

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
- Dallos, R., & Draper, R. (2015). An Introduction to Family Therapy (4th ed.). Open University Press.

Foundations and history

- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Bateson, G. (1972). Steps to an Ecology of Mind. Chandler.
- Hoffman, L. (1981). Foundations of Family Therapy. Basic Books.
- Gurman, A. S., Lebow, J. L., & Snyder, D. K. (Eds.). (2015). Clinical Handbook of Couple Therapy (5th ed.). Guilford.

Practice and technique

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Haley, J. (1980). Leaving Home. McGraw-Hill.
- McGoldrick, M., Gerson, R., & Petry, S. (2020). Genograms: Assessment and Treatment (4th ed.). Norton.

Culture and critique

- McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.). Guilford.
- Hare-Mustin, R. (1978). A feminist approach to family therapy. Family Process, 17(2), 181 194.

Evidence

- Wampold, B. E., & Imel, Z. E. (2015). The Great Psychotherapy Debate (2nd ed.). Routledge.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Shadish, W. R., & Baldwin, S. A. (2003). Effects of family and marital psychotherapies: A meta-analysis. Journal of Consulting and Clinical Psychology, 71(6), 1080 1097.

Codes and standards

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- International Association of Marriage and Family Counselors. Ethical Code.

Next: MFT 702 Theories of Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 702

Theories of Family Therapy

14 teaching weeks · 8 practicals · 17 worked cases · 9 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701

> “Nine major schools, each treated as an answer to a specific question.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0 · 2026 edition

MFT 702 — Theories of Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 702 — Theories of Family Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 702	**Theories of Family Therapy**	One	1.71

Contents

How to use this book . . . 15

1. Course description . . . 15

2. Learning outcomes . . . 15

3. Assessment . . . 15

4. The fourteen-week plan . . . 16

Week 1 — How to compare theories: six dimensions . . . 16

1.1 The lecture in ninety minutes . . . 16

1.2 Why comparison is a skill . . . 17

1.2a How to study nine models in fourteen weeks . . . 18

1.3 The six dimensions . . . 18

1.4 Theory of problem as the master dimension . . . 20

1.4a The six dimensions as questions you can ask a family . . . 20

1.5 Three levels of knowing a model . . . 21

1.6 Comparing three models on one family . . . 22

1.6a The matrix as a clinical instrument: choosing under pressure . . . 23

1.7 The reduction test and the charity test . . . 24

1.8 The same ten minutes, three times . . . 24

1.9 Laboratory: building the matrix . . . 26

1.9a An annotated matrix row, and a row that fails . . . 27

1.10 Six errors in comparing models . . . 28

1.10a The viva question bank . . . 29

1.11 Culture, faith and the comparative gaze . . . 29

1.11a Where each model's theory of problem came from . . . 30

1.12 Evidence note: what comparison can and cannot establish . . . 31

1.13 Key terms . . . 31

1.13a Two student matrices, and what the marker sees . . . 32

1.14 Self-test . . . 33

1.15 Reflective task . . . 33

1.16 Readings for this week . . . 33

Week 2 — Psychoanalytic and object-relations family therapy . . . 34

2.1 The lecture in ninety minutes . . . 34

2.2 Core concepts . . . 34
2.2a Four concepts, one family . . . 35
2.3 Therapy: the shape and the mechanism . . . 36
2.3a How long is this therapy, and who can have it? . . . 37
2.4 Signature technique: interpretation, used sparingly . . . 38
2.5 Contribution, and what the field kept . . . 38
2.5a The Tavistock tradition and group-analytic family therapy . . . 39
2.6 Blind spots, stated precisely . . . 40
2.6a What the school got wrong, in its own history . . . 40
2.7 Annotated transcript: an interpretation that lands, and one that does not . . . 41
2.7a What the therapist brings: the use of self in a slow model . . . 43
2.8 Laboratory: finding the projection . . . 43
2.8a The countertransference log . . . 44
2.9 Six errors in object-relations family work . . . 45
2.9a Two sessions with the same family, one object-relations and one not . . . 45
2.10 Culture, faith and the unconscious . . . 46
2.10a Working with a family whose culture does not use the model's vocabulary . . . 47
2.11 Evidence note: what is actually known . . . 48
2.12a A reading exercise: three paragraphs of Bion . . . 48
2.12 Key terms . . . 49
2.13a The model in three sentences, and what each sentence costs . . . 49
2.13 Self-test . . . 50
2.14 Reflective task . . . 51
2.15 Readings for this week . . . 51
Week 3 — Bowen's multigenerational theory . . . 51
3.1 The lecture in ninety minutes . . . 52
3.2 Core concepts . . . 53
3.2a The eight concepts as one process . . . 54
3.3 Differentiation, tested . . . 55
3.3a Four families, one anxiety: predicting who absorbs it . . . 55
3.4 Triangles, and how to see one . . . 56
3.4a The matrix row, written . . . 57
3.5 Therapy: the coach, not the fixer . . . 58
3.5a Coaching, demonstrated in five exchanges . . . 58

3.6 Signature technique: detriangling . . . 59
3.6a Six experiments in contact, and how to design a seventh . . . 60
3.7 Annotated transcript: detriangling in the room . . . 61
3.8 Laboratory: three moves, three rounds . . . 62
3.8a The supervision question that keeps the coach honest . . . 63
3.9 Six errors in Bowen work . . . 63
3.9a Two genograms, one pattern . . . 64
3.10 Culture, faith and the multigenerational frame . . . 65
3.11 Evidence note: clinical theory, thin trials . . . 65
3.11a Where Bowen and structural disagree, on one family . . . 66
3.12 Key terms . . . 67
3.13 Self-test . . . 68
3.14 Reflective task . . . 68
3.15 Readings for this week . . . 69
Week 4 — Structural family therapy . . . 69
4.1 The lecture in ninety minutes . . . 69
4.2 Core concepts . . . 70
4.2a Reading a family's structure in ten minutes . . . 71
4.3 Therapy: join, then challenge . . . 72
4.3a Joining, technique by technique . . . 72
4.4 Signature techniques . . . 73
4.5 The structural first session . . . 74
4.5a The structural map, drawn and used . . . 75
4.6 The Adeyemi first move, worked in full . . . 76
4.6a Two structural first sessions, one that works and one that does not . . . 77
4.7 Annotated transcript: an enactment that works . . . 77
4.7a When the enactment fails, in three ways . . . 79
4.8 Laboratory: one enactment, three attempts . . . 79
4.8a The matrix row, written . . . 80
4.9 Six errors in structural work . . . 81
4.10 Culture, faith and the structural norm . . . 81
4.10a Two critiques, stated in their strongest form . . . 82
4.11 Evidence note: the strongest evidence among the classical models . . . 83
4.11a Structural work with a family in which the father is absent by circumstance . . . 84

4.12 Key terms . . . 85
4.13 Self-test . . . 85
4.13a The task, and the four tests of a good one . . . 86
4.14 Reflective task . . . 87
4.15 Readings for this week . . . 87
Week 5 — Strategic and MRI brief therapy . . . 87
5.1 The lecture in ninety minutes . . . 87
5.1a The Palo Alto context, in five sentences . . . 88
5.2 Core concepts . . . 88
5.2a Six attempted solutions that recur . . . 89
5.3 Therapy: the active strategist . . . 90
5.3a Listening for the attempted solution . . . 91
5.4 Signature techniques . . . 92
5.4a A reframe, built in four steps . . . 92
5.5 Paradox, and its ethics . . . 93
5.6 The brief architecture: a five-session frame . . . 94
5.6a The five-session frame, worked through one case . . . 95
5.7 The Adeyemi loop, traced . . . 96
5.7a The matrix row, written . . . 96
5.8 Annotated transcript: building a directive with a reluctant father . . . 97
5.9 Laboratory: design one directive . . . 98
5.9a Three directives that failed, and why . . . 99
5.10 Six errors in strategic work . . . 100
5.11 Culture, faith and the strategic stance . . . 100
5.11a Where the strategic and the Milan schools part company . . . 101
5.12 Evidence note: what the trials support . . . 102
5.12a The ethics of direction, stated as five rules . . . 102
5.13 Key terms . . . 103
5.14 Self-test . . . 103
5.15 Reflective task . . . 104
5.16 Readings for this week . . . 104
Week 6 — Milan systemic therapy . . . 105
6.1 The lecture in ninety minutes . . . 105

6.2 Core concepts . . . 106
6.2a The family premise, found in three questions . . . 107
6.3 The three-part stance . . . 107
6.4 Positive connotation, and its logic . . . 108
6.4a A positive connotation, built line by line . . . 109
6.5 Circular questioning . . . 110
6.5a Twenty circular questions, grouped . . . 111
6.6 Annotated transcript: circular questions in a first session . . . 112
6.7 Neutrality, and its ethical limit . . . 113
6.7a Neutrality in a crisis: a three-minute script . . . 114
6.8 The reflecting team, and the modern non-directive version . . . 114
6.8a A reflecting team, run with three trainees . . . 115
6.9 Laboratory: ten minutes of circular questions . . . 116
6.10 Six errors in Milan work . . . 117
6.10a Three circular interviews that failed . . . 117
6.11 Culture, faith and the Milan stance . . . 118
6.11a Hypothesising about a family's faith, without appropriating it . . . 119
6.12 Evidence note: a stance rather than a treatment . . . 120
6.13 Key terms . . . 120
6.14 Self-test . . . 121
6.15 Reflective task . . . 122
6.16 Readings for this week . . . 122
Week 7 — Experiential and human validation models . . . 122
7.1 The lecture in ninety minutes . . . 123
7.1a Whitaker's absurdity, demonstrated three ways . . . 124
7.2 Core concepts . . . 125
7.2a Two Satir maps, learned by drawing them . . . 126
7.3 The therapy: warmth, pace and the person of the therapist . . . 127
7.3a Where the therapist's own history enters the room . . . 128
7.4 Techniques, with the grammar of each . . . 128
7.4a A sculpture, drawn and then read . . . 129
7.5 The first session: contact, safety and the first invitation . . . 130
7.5a A first-session opening, scripted in eleven sentences . . . 131
7.6 Case: the Adeyemis and the unsaid sentence . . . 131

7.6a The same family, with the therapy done badly . . . 132
7.7 Annotated transcript: one enactment, minute by minute . . . 133
7.8 Laboratory: the four-move enactment . . . 134
7.9 Six errors in experiential work . . . 135
7.10 Culture, faith and the model's critique . . . 136
7.10a The two strongest critiques, and the model's answers . . . 137
7.11 Evidence note: what is known and what is claimed . . . 137
7.12 Key terms . . . 138
7.13 Self-test . . . 138
7.14 Reflective task . . . 139
7.15 Readings for this week . . . 139
Week 8 — Cognitive-behavioural and behavioural couple therapy . . . 140
8.1 The lecture in ninety minutes . . . 141
8.2 Core concepts . . . 142
8.2a Two behavioural ideas, misread and corrected . . . 144
8.3 The therapy: a structured course, session by session . . . 144
8.3a The first three sessions in detail, with the words . . . 145
8.4 Techniques, specified . . . 146
8.5 The first session with a couple who have stopped speaking . . . 147
8.5a A behavioural exchange contract, designed and then criticised . . . 147
8.6 Case: the Adeyemis, in behavioural form . . . 149
8.6a The same couple in a behavioural formulation, and in a systemic one . . . 149
8.7 Annotated transcript: attribution retraining with a couple . . . 150
8.7a Speaker–listener, three ways it goes wrong . . . 152
8.8 Laboratory: design, rehearse, review . . . 152
8.9 Six errors in behavioural couple work . . . 153
8.10 Culture, faith and the model's critique . . . 154
8.10a The two strongest critiques, and the model's answers . . . 154
8.10b Faith, and the limits of a technique . . . 155
8.11 Evidence note: the strongest base in the course . . . 156
8.12 Key terms . . . 156
8.13 Self-test . . . 157
8.14 Reflective task . . . 158
8.15 Readings for this week . . . 158

Week 9 — Emotionally focused therapy . . . 158
9.1 The lecture in ninety minutes . . . 160
9.2 Core concepts . . . 161
9.2a A map of the cycle, drawn in four boxes . . . 162
9.2b Attachment strategies, and how they present in a couple session . . . 163
9.3 The therapy: three stages, and what happens in each . . . 164
9.3a Stage two in slow motion: the withdrawer's re-engagement . . . 165
9.4 Techniques, with the grammar of each . . . 166
9.4a A deepening sequence, scripted in eight turns . . . 166
9.5 The first session with Aisha and Sam . . . 167
9.6 Case: the Adeyemis, read through attachment . . . 168
9.7 Annotated transcript: the softening event . . . 169
9.7a Three softenings that failed, and why . . . 170
9.8 Laboratory: deepening and enactment . . . 171
9.9 Six errors in EFT practice . . . 171
9.9a The couple who arrive at stage two too early . . . 172
9.10 Culture, faith and the model's critique . . . 172
9.10a The two strongest critiques, and the model's answers . . . 173
9.11 Evidence note: the strongest couple evidence in the course . . . 174
9.12 Key terms . . . 175
9.13 Self-test . . . 175
9.14 Reflective task . . . 176
9.15 Readings for this week . . . 176
Week 10 — Solution-focused brief therapy . . . 177
10.1 The lecture in ninety minutes . . . 178
10.2 Core concepts . . . 179
10.2a Two transcripts of the same first ten minutes . . . 180
10.2b The questions this model is actually made of . . . 181
10.3 The session, in five moves . . . 182
10.3a The miracle question, built question by question . . . 182
10.4 Techniques, with the grammar of each . . . 183
10.4a A scaling exercise, worked in full . . . 184
10.5 The first session with the Adeyemis . . . 185

10.6 Case: what the model did not ask, and the session after . . . 186
10.7 Annotated transcript: the miracle question with a couple . . . 187
10.8 Laboratory: the miracle question and the scale . . . 188
10.8a A task designed for a family that has heard everything . . . 189
10.9 Six errors in solution-focused work . . . 190
10.9a Three sessions where the model was the wrong choice . . . 190
10.10 Culture, faith and the model's critique . . . 191
10.10a The two strongest critiques, and the model's answers . . . 192
10.11 Evidence note: real, uneven, and often oversold . . . 192
10.12 Key terms . . . 193
10.13 Self-test . . . 194
10.14 Reflective task . . . 194
10.15 Readings for this week . . . 195
Week 11 — Narrative therapy . . . 195
11.1 The lecture in ninety minutes . . . 197
11.2 Core concepts . . . 197
11.2a An externalising conversation, line by line . . . 199
11.3 The therapy: what a narrative session looks like . . . 200
11.3a Mapping the influence, in two columns . . . 201
11.3b The notebook, and why it is a technique . . . 202
11.4 Techniques, with the grammar of each . . . 202
11.5 The first session with a family who have been described . . . 203
11.6 Case: the mother described as negligent . . . 204
11.7 Annotated transcript: re-authoring with a young person . . . 205
11.7a Three externalising conversations that failed . . . 207
11.8 Laboratory: externalising, mapping and a letter . . . 207
11.9 Six errors in narrative work . . . 208
11.9a What the model does with a diagnosis . . . 208
11.10 Culture, faith and the model's critique . . . 209
11.10a The two strongest critiques, and the model's answers . . . 210
11.11 Evidence note: promising, thin, and honest about it . . . 211
11.12 Key terms . . . 211
11.13 Self-test . . . 212
11.14 Reflective task . . . 213

11.15 Readings for this week . . . 213
Week 12 — Integration without incoherence . . . 213
12.1 The lecture in ninety minutes . . . 215
12.2 Why every practitioner integrates, and what goes wrong . . . 215
12.2a A formulation that travels, and one that does not . . . 216
12.3 The three forms, described properly . . . 217
12.3a The five rules, applied . . . 219
12.4 Choosing: the decision tree . . . 220
12.4a A worked integration: the Adeyemis, five phases . . . 220
12.5 The handover: how to change model without losing the family . . . 222
12.5a The first two sessions of an integrated therapy . . . 222
12.6 Case: the same family, integrated badly . . . 223
12.6a When to stop integrating and refer . . . 224
12.7 Annotated transcript: the review session . . . 225
12.7a Two integrations that should not have been attempted . . . 226
12.8 Laboratory: the integration audit . . . 226
12.9 Six errors in integration . . . 227
12.10 Culture, faith and the ethics of integration . . . 228
12.10a What the evidence says about integration . . . 229
12.11 Key terms . . . 229
12.12 Self-test . . . 230
12.13 Reflective task . . . 230
12.14 Readings for this week . . . 230
Week 13 — Evidence, culture and critique . . . 231
13.1 The lecture in ninety minutes . . . 231
13.2 The matrix, completed and read . . . 232
13.3 What counts as evidence, ranked . . . 233
13.4 The common factors, and the variance argument . . . 234
13.4a The four levels of evidence in routine practice . . . 234
13.5 Whose family? The sample problem . . . 235
13.6 Translating a model, worked three times . . . 236
13.6a A translation that went wrong . . . 238
13.7 Case: the family the trials never saw . . . 239

13.7a Two critiques of this course's own matrix . . . 240
13.8 Laboratory: the four-minute defence . . . 241
13.8a A cultural translation checklist . . . 241
13.9 Annotated transcript: a viva, examined . . . 242
13.10 Six errors in the evidence and culture week . . . 244
13.10a Faith, and the evidence question . . . 244
13.10b What a critique is for . . . 245
13.11 Evidence note: how to write an evidence claim . . . 246
13.12 Key terms . . . 246
13.13 Self-test . . . 247
13.14 Reflective task . . . 248
13.15 Readings for this week . . . 248
Week 14 — Your theoretical position . . . 248
14.1 The lecture in ninety minutes . . . 249
14.2 The three components of a position . . . 249
14.2a Three positions, defended . . . 250
14.3 Borrowing with conditions . . . 252
14.4 The change-of-mind clause . . . 253
14.5 Annotated transcript: the viva . . . 254
14.6 The statement: an annotated exemplar . . . 256
14.6a Four statements that fail, and why . . . 258
14.7 Laboratory: the position, attacked . . . 258
14.8 Six errors in constructing a position . . . 259
14.9 Culture, faith and the position . . . 259
14.9a Two critiques of the position project, and the replies . . . 261
14.10 Annotated transcript: the position after two years of practice . . . 261
14.11 Laboratory: the final rehearsal . . . 262
14.12 Evidence note: what this week can and cannot evidence . . . 263
14.13 Key terms . . . 264
14.14 Self-test . . . 264
14.15 Reflective task . . . 265
14.16 Readings for this week . . . 265
Integration: nine models on one page . . . 266

Glossary 267
Self-assessment 267
Multiple choice 267
Answer key 268
Short answer 268
Essay (choose one, 2,000 words) 268
Further reading 268

Course leader's summary. Nine major schools, each treated as an answer to a specific question. You will learn each model well enough to do a first session in it, critique it, and integrate it — and you will practise moving between models on the same family until the choice becomes deliberate rather than habitual.

How to use this book

Theory is learned in three passes. **Pass one:** understand the model's core question and its intervention. **Pass two:** apply it to a case you know. **Pass three:** critique it and ask what it cannot see. Each week in this book gives you all three, plus the practical that makes the pass real. Do not memorise lists of techniques; memorise what problem each model believes it is solving.

1. Course description

There is no single "correct" family therapy. There are traditions, each with a theory of how problems develop, a theory of change, a preferred focus of attention, and a characteristic set of techniques. A clinician who knows only one model does not have a theory; they have a habit.

This course therefore pursues three aims: **comparative competence** (you can describe and demonstrate each major model), **critical appraisal** (you can name each model's evidence base and its blind spots), and **integration** (you can combine two models coherently in one case rather than dithering between them).

Organising question. For each model, ask: What does this model believe is the problem, and what does it therefore do first?

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the theoretical foundations, core concepts and change mechanisms of nine major models	Exam
LO2	Demonstrate a model-consistent first session for at least three models	Skills assessment
LO3	Compare models on focus, technique, role of the therapist and evidence	Comparative matrix
LO4	Identify the blind spots and cultural assumptions of each model	Essay
LO5	Construct a coherent integrative plan for a single case using two models	Case assignment
LO6	Justify model choice with reference to the client, the evidence and your own competence	Viva

3. Assessment

Component	Weight	Due
Comparative matrix (nine models × six dimensions)	20%	Week 7
Observed model-consistent session (20 min)	20%	Weeks 8–10
Integrative case study (2,500 words)	30%	Week 12
Written examination	30%	Week 15

4. The fourteen-week plan

Week	Model / topic	Practical	Key text
1	How to compare theories: six dimensions	Matrix construction	Nichols ch. 1
2	Psychoanalytic and object-relations family therapy	Genogram + interpretation exercise	Scharff & Scharff
3	Bowen's multigenerational theory	Detriangling role-play	Bowen (1978)
4	Structural family therapy	Enactment and boundary-making	Minuchin (1974)
5	Strategic and MRI brief therapy	Directive design	Haley & Madanes
6	Milan systemic therapy	Circular questioning	Palazzoli et al. (1980)
7	Experiential and human validation (Satir, Whitaker)	Sculpture exercise	Satir (1988)
8	Cognitive-behavioural and behavioural couple therapy	Behavioural exchange design	Epstein & Baucom
9	Emotionally focused therapy	EFT enactment	Johnson (2019)
10	Solution-focused brief therapy	Miracle question and scaling	de Shazer (1985)
11	Narrative therapy	Externalising conversation	White (2007)
12	Integration: combine two models without incoherence	Case integration workshop	Lebow (2014)
13	Evidence, culture and critique of each model	Debate	Sprenkle et al.
14	Your theoretical position	Position paper presentation	—

Week 1 — How to compare theories: six dimensions

Every model in this course will be described on the same six axes. Learn to fill the grid from memory and you will be able to enter any family therapy conversation — an interview, a case conference, a viva — with something more useful than an opinion. Learn the models as nine separate stories and you will have nine stories and no method.

1.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The problem with model surveys	Two clinicians, one family, incompatible first questions	Predict what each will do in the first five minutes
10–25	The six dimensions	Define each, with a counter-example	Fill two rows of the matrix from memory
25–40	Theory of problem as master dimension	Show how the other five follow from it	Derive a model's techniques from its theory of problem
40–55	Three levels of knowing a model	Describe, demonstrate, critique	Rate themselves on each level for one model
55–70	Comparing three models on one case	Fill the grid live from the Adeyemi material	Add a fourth model
70–85	The reduction test and the charity test	Apply both to a claim about a model	Test two claims in pairs
85–90	Set the assessment	The matrix, its due date and its marking	Plan their matrix file

1.2 Why comparison is a skill

A clinician who knows one model does not have a theory; they have a habit. That sentence is the premise of the course, and this week explains why it is true and what to do about it.

What comparison is for. Four uses, each practical.

First, it makes the first session deliberate. The first five minutes of a family session are shaped by a model whether or not the therapist has chosen one. The structural therapist moves to the hierarchy; the narrative therapist asks what name the family would give the problem; the emotionally focused therapist watches for the cycle. Comparison allows the therapist to know which of these they are doing, and to choose.

Second, it protects against the single-story error. Every model has a characteristic blind spot, and the blind spot is usually the same as its strength. A therapist who cannot name the blind spot of their preferred model cannot see the family that the model is failing.

Third, it makes referral and supervision possible. Knowing that a family needs something you do not do is the beginning of professional judgement. Knowing it in the vocabulary of the field — "this family needs a structural first phase and then narrative work" — makes the referral legible to a colleague.

Fourth, it is examinable, and it is examined everywhere. The comparative matrix, the viva, the case study: all three assessments in this course are applications of the six dimensions. Students who build the matrix properly in Week 1 are doing the exam in October.

What comparison is not. It is not a listing of techniques, a history of ideas, or an argument about which model is best. It is a method for answering a specific question about any model: what does this model believe the problem is, and what does it therefore do first?

1.2a How to study nine models in fourteen weeks

The course is short and the field is large, so the method matters as much as the effort. Five practices distinguish students who can use the models from students who can list them.

Practice 1 — one primary text per model, chosen for size. Not the complete works; a single book or long chapter that the model's author wrote for practitioners. For each model in this course, the reading list gives one such text. Read it in the week, at speed, for the model's own vocabulary; the secondary summaries will then have something to attach to.

Practice 2 — the one-sentence discipline. At the end of each reading session, write the theory of problem in one sentence, in the author's terms, and then write a second sentence saying what the author believes is not the problem. The second sentence is what distinguishes models, because every model defines itself against a predecessor.

Practice 3 — apply the model to a family you know. Not a hypothetical family: the Adeyemi case, or a family from placement, or the running case in the seminars. Each week, write one page applying the new model to the same family. By Week 11 you will have eleven pages on one family, which is the best comparative document you will ever own, because the family is held constant and only the lens changes.

Practice 4 — record yourself at level 2 once per model. Twenty minutes with a role-played family, in the model's terms, recorded. Watch it with the model's signature techniques written in front of you and count how often you used them. Most students find that they used the techniques of the model they learned first, whatever model they were attempting, which is the clearest possible demonstration of why deliberate practice exists.

Practice 5 — keep a disagreement file. Every time a model's claim conflicts with another's, or with the evidence, or with your own experience, write the conflict down in two sentences with the date. Bring the file to the seminars. The disagreements are the material of the essay, and they are what the viva will test.

What the practices have in common. They all convert reading into something that can be observed: a sentence, a page, a recording, a note. Reading alone produces the illusion of knowledge that the first seminar exposes, and the practices are designed to expose it earlier, in private, where it costs nothing.

1.3 The six dimensions

The grid is the spine of the course. Each dimension is a question with a determinate answer for each model, and the answers are not independent: the first dimension generates most of the others.

Dimension	The question it answers
Theory of problem	How does this model believe problems develop and persist?
Unit of focus	Individual, dyad, family structure, meaning system, or context?
Theory of change	What must happen for things to improve?

Dimension	The question it answers
Therapist position	Director, coach, collaborator, witness, expert?
Signature techniques	What does this model characteristically do?
Evidence base	What is the strength and quality of supporting research?

Dimension 1 — theory of problem. How does this model believe problems develop and persist? This is the master dimension. A model that believes problems are maintained by structure will intervene in structure; a model that believes they are maintained by meaning will intervene in meaning; a model that believes they are maintained by unexpressed emotion will intervene in emotion. Everything else follows, and a student who cannot state the theory of problem in one sentence has not learned the model.

Dimension 2 — unit of focus. What is the model looking at? The individual in context, the dyad, the triadic or hierarchical structure, the family's meaning system, the multigenerational transmission, or the wider context of institutions and culture. The unit determines what counts as data and what counts as noise. To the structural therapist, a mother's tearfulness is data about the family's organisation; to the emotion-focused therapist, the organisation is data about the tearfulness.

Dimension 3 — theory of change. What must happen for things to improve, and in what order? Some models believe insight precedes change; some that change precedes insight; some that a small difference is sufficient; some that the system must be destabilised before it can reorganise. This dimension predicts the shape of the therapy — its length, its pacing, and what the therapist will do when nothing is happening.

Dimension 4 — therapist position. Director, coach, collaborator, witness, expert, non-expert, consultant. The position determines who speaks, who decides, and what the family is told about what is happening. It is the dimension most visible to families, and the one trainees most often adopt by accident.

Dimension 5 — signature techniques. What does this model characteristically do? Enactment, reframing, circular questioning, genogram, sculpture, externalising, miracle question, behavioural exchange, two-chair work. Techniques are the easiest dimension to learn and the least useful on their own, because a technique divorced from its theory of problem is a gesture.

Dimension 6 — evidence base. What is the strength and quality of the research support, for what presentations, and with what limits? This dimension is the one most often omitted in textbooks and most likely to be tested in a viva, because a clinician's claim about evidence is a claim about what they owe a family.

Two further dimensions worth adding to the matrix. The six are the required grid; two more make it a professional instrument.

Dimension 7 — cultural assumptions. Every model carries a picture of a healthy family, and that picture has a culture. A model that prizes individuation will read a three-generational household differently from a model that prizes interdependence. The

question is not whether the model has cultural assumptions — all do — but what they are, and what they make the model unable to see.

Dimension 8 — the characteristic blind spot. What does this model systematically not notice? The blind spot is not a defect to be apologised for; it is the shadow of the model's strength, and it is the most useful thing a comparative therapist knows.

1.4 Theory of problem as the master dimension

The claim that the first dimension generates the others is testable in an exercise that takes ten minutes and is worth a term of reading.

The exercise. Write down, in one sentence, what a model believes maintains the problem. Then derive, without looking anything up, what its unit of focus must be, what its theory of change must be, what position its therapist must occupy, and two techniques it must use.

Worked derivation 1 — structural. Theory of problem: difficulties are maintained by the family's organisation — unclear hierarchies, inappropriate boundaries, cross-generational coalitions. Unit of focus: the whole family as a structure, with subsystems. Theory of change: the structure must change in the room, through action, before understanding follows. Therapist position: active director, at times stage-manager. Techniques: enactment, unbalancing, boundary-making, spatial rearrangements.

Worked derivation 2 — narrative. Theory of problem: difficulties are maintained by the dominant stories that families and institutions hold about their members, and by the practices those stories make possible. Unit of focus: the meaning system, and the social context in which meanings circulate. Theory of change: the story must be made visible, the person separated from the problem, and a preferred account thickened. Therapist position: collaborative, curious, decentred but influential. Techniques: externalising, re-authoring, unique outcomes, definitional ceremonies.

Worked derivation 3 — emotion-focused. Theory of problem: distress is maintained by rigid negative interactional cycles that organise around unexpressed primary emotion and unmet attachment needs. Unit of focus: the couple's cycle, and the attachment bond that the cycle protects. Theory of change: the cycle must be slowed, the primary emotion accessed and expressed, and a new cycle enacted in the room. Therapist position: active, empathic, a process consultant who stages the encounter. Techniques: tracking the cycle, reframing in terms of the cycle, the enactment, and the "hold me tight" conversation.

What the derivations demonstrate. That a model is a system, not a list. Students who can generate the five other dimensions from the first have understood a model; students who have memorised six columns have not. The examination will ask for a derivation, in some form, every year.

1.4a The six dimensions as questions you can ask a family

The grid is usually taught as an instrument for reading textbooks. It is more useful as an instrument for the room, because each dimension can be converted into a question that changes what the family tells you.

From theory of problem to a question about maintenance. Model reading: the model believes something specific maintains the problem. Question: "What keeps this going, do you think? Not how it started — what keeps it going now?" Families answer this question with a mechanism, and the mechanism they describe is often more accurate than the formulation in the referral.

From unit of focus to a question about who matters. Question: "Who else is affected by this — and who else has an opinion about it?" In families where the meaningful unit is wider than the household, this question recruits the grandmother, the aunt or the pastor into the conversation before the therapist has decided they are relevant.

From theory of change to a question about what has helped before. Question: "When it has been better, even for a week, what made the difference?" This question tests the model's theory of change against the family's own experience: if the family's answer is "we agreed one thing and stuck to it", a structural first move is indicated; if it is "we stopped talking about it and it faded", a solution-focused approach has better odds.

From therapist position to a question about what the family wants from you. Question: "Some families want me to tell them what to do; others want me to help them work it out. Which would be more useful to you?" The answer is not binding, and asking it prevents the commonest early rupture: a family expecting direction receiving exploration, or the reverse.

From signature technique to a question about what the family has already tried. Question: "What have you tried, and what happened?" This is the shortest route to the model's technique, because the family's failed solutions are usually the inverse of what the model would do.

From evidence base to a question about the family's own data. Question: "Would you be willing to rate how things are each week, so we can see whether this is working rather than guessing?" The evidence dimension made operational.

The point of the exercise. A dimension that cannot generate a question is a dimension the student has learned verbally. The test of understanding a model is not that you can describe its unit of focus but that you know what to ask a family in order to work with it.

1.5 Three levels of knowing a model

A student can be at three different levels with the same model, and the levels are assessed separately.

Level 1 — describe. You can state the theory of problem, the unit of focus, the theory of change, the therapist position, two or three techniques and the evidence base, in your own words, without notes. This is the level required for the written examination, and it is the level at which most textbooks stop.

Level 2 — demonstrate. You can conduct twenty minutes of a model-consistent session with a role-played family, so that an observer who knows the model can identify it from the transcript without being told. This is the level required for the observed session, and it

is much harder than describing, because it requires knowing what the model sounds like: the actual sentences, the pacing, the shape of the first ten minutes.

Level 3 — critique. You can state the model's blind spot, its cultural assumptions, the presentations for which the evidence is strong and weak, and what you would do when the model fails. This is the level required for the essay and the viva, and it is the level at which a clinician becomes safe to practise, because it is the level at which they know when to stop.

The diagnostic. For each model in this course, rate yourself 0–3 on each level: 0 (cannot), 1 (with notes), 2 (reliably), 3 (can teach it). Bring the grid to the Week 7 practical. Most students overrate level 1 and underrate level 2, and the pattern is worth knowing about oneself.

Why the levels matter for the matrix. The comparative matrix is a level 1 instrument. It is necessary and it is not sufficient. A student who can fill the matrix and cannot demonstrate a single model has learned a taxonomy, not a therapy.

1.6 Comparing three models on one family

The Adeyemi family, from MFT 701, is the running case. The grid below is filled for three models, and the fourth column is the week's exercise.

Dimension	Structural	Bowen	Solution-focused
Theory of problem	Maintained by unclear hierarchy, diffuse or rigid boundaries, cross-generational coalitions	Maintained by anxiety transmitted across generations and absorbed by the most vulnerable member; triangles	Maintained by a problem-saturated description and by the family's attention to what is not working
Unit of focus	Whole family as a structure, with subsystems and boundaries	Multigenerational emotional system; triangles; differentiation	The family's description of the problem and its preferred future
Theory of change	Restructure in the room; action before insight	Lower reactivity, raise differentiation, detriangle	Build a description of the solution and amplify existing exceptions
Therapist position	Active director, stage-manager	Coach, neutral observer of process	Curious collaborator, expert in process not in lives
Signature techniques	Enactment, unbalancing, boundary-making	Genogram, coaching, detriangling questions	Miracle question, scaling, exception-seeking
Evidence base	Moderate: strong for adolescent behaviour problems and eating disorders in adapted forms	Thin: influential, widely taught, little trial evidence	Moderate for depression and behaviour problems in brief formats; strong evidence for the components (goal setting, scaling)
Cultural assumption	A family should have a clear hierarchy with parents in charge	Differentiation is a good; the individual should think for themselves	The family can describe a preferred future and act on it

Dimension	Structural	Bowen	Solution-focused
Blind spot	Can impose a structure the family experiences as foreign; may miss meaning, loss and material conditions	Slow; may intellectualise; may under-attend to practical and institutional pressure	May bypass the function of the symptom and the history that sustains it

What the grid shows at a glance. Three competent clinicians, one family, three different first sessions — and none of them is wrong. The grid also shows why model loyalty is a temperamental matter: the structural column appeals to therapists who want to act, the Bowen column to therapists who want to understand, the solution-focused column to therapists who want to build. The professional question is not which temperament is best but which the family needs.

The fourth column. Fill it for narrative therapy before the seminar, from the reading, and bring it. The exercise is deliberately placed before the seminar so that students discover what they cannot yet do.

1.6a The matrix as a clinical instrument: choosing under pressure

In a real clinic the matrix is used in ninety seconds, in a corridor, with a referral in your hand. Three ways it earns its keep.

Case 1 — the referral that does not fit your model. A family is referred for a child's behaviour problems. You work structurally. The referral letter says the parents separated two years ago, the child sees the father fortnightly, and the mother reports that the boy "is fine with his father and impossible with me". The matrix question is: whose unit of focus does this fit? The structural lens would look at the mother–son subsystem; the matrix reminds you that the child's split presentation across two households is a different unit, and that a model with a place for triangulation across households (or a systemic model that treats the two households as one system) may be indicated. The matrix has not told you what to do; it has stopped you from doing the obvious thing.

Case 2 — the family that has already had two therapies. A couple arrives having had behavioural couple therapy (which they found mechanical) and then six sessions of a psychodynamic approach (which they found slow). The matrix question is: what theory of change have they already tested and rejected, and what remains? A therapist who knows the grid can offer a third mechanism deliberately — an emotion-focused or narrative approach — rather than repeating what has failed, and can say why: "You have tried changing what you do and you have tried understanding where it came from. There is a third route: changing what happens between you in the moment."

Case 3 — the model you cannot deliver. A family presents with an adolescent's anorexia. You are trained in structural and narrative work and not in family-based treatment. The matrix's evidence column says the specific evidence is strongest for a model you do not practise. The professional response is visible in the grid: refer, or co-work, or seek training — and say so to the family. The matrix is at its most useful when it tells you that you are not the right clinician.

What all three cases share. The matrix is not a decision procedure that produces an answer. It is a discipline that widens the options visible to a therapist under pressure, which is where most clinical errors are made.

1.7 The reduction test and the charity test

Two habits of thought that prevent most of the errors in this course.

The reduction test. When you are tempted to say that a model is "just" something — "structural therapy is just common sense about who is in charge", "narrative therapy is just reframing with better marketing" — apply the reduction test: does the reduction preserve the model's distinctive claim about what maintains the problem? If not, the reduction is a dismissal in disguise. Structural therapy's claim is not that hierarchy is obvious; it is that hierarchy is enacted in the room, in the present, in observable space, and that changing the enactment changes the family. That claim is not common sense, and it can be tested.

The charity test. When you are tempted to defend a model against a criticism, apply the charity test: state the criticism in its strongest form, in the critic's own terms, before answering it. A model defended against a weak version of the objection has not been defended. This is the discipline that the essay assessment rewards, and it is the discipline that distinguishes a professional from a partisan.

How the two tests work together. The reduction test stops you dismissing a model too quickly; the charity test stops you defending one too quickly. Between them they produce the posture this course is trying to teach: interested, informed, uncommitted to a conclusion, and able to act.

> **PRACTICAL**
> Take one criticism of each of the three models in 1.6 — for example, "structural therapy is manipulative", "Bowen theory is untestable", "solution-focused therapy ignores suffering". State each in its strongest form in two sentences, then write the best available answer in two sentences. Bring all six sentences to the seminar.

1.8 The same ten minutes, three times

A transcript exercise that makes the grid concrete. The family is the Adeyemis; the first ten minutes are written three times, in the voice of three models, and the differences are marked.

Version A — structural.

> **Therapist:** Before we start, I want to know who is in charge of the school question in this house, because somebody has to be. Mrs Adeyemi, when the school telephones, who decides what happens? **Mother:** I do. He is not here.
> **Therapist:** And when he is here, at the weekend? **Mother:** Then he decides, and then he tells me I have done it wrong.
> **Therapist:** So in this house there are two governments, and Amara lives in both of them. (to Amara) Which one do you go to first? **Amara:** My mum. She says yes.
> **Therapist:** (to the parents) I want the two of you to sit together for the next ten minutes and decide one thing about Monday, while Amara and I listen. Not negotiate — decide.

What this version does. It establishes the hierarchy question in the first ninety seconds, names the coalition, addresses the child directly with a question that reveals the coalition, and moves immediately to an enactment. It uses space and action. Note what is absent: no history, no feelings, no exploration of what the family has tried.

Version B — Bowen.

> **Therapist:** I want to start with each of you, one at a time, and I want to ask about the anxiety in this house rather than about the school. Mrs Adeyemi, when Amara says she is not going, what happens inside you? **Mother:** I panic. My chest. Then I am angry.
> **Therapist:** And when you panic, what do you do? **Mother:** I telephone the school. I fix it. I have always fixed things.
> **Therapist:** You have always fixed things. Who taught you that? **Mother:** (pause) Nobody taught me. I had to.
> **Therapist:** And Mr Adeyemi, when you come home on Friday and hear about the week, what happens inside you? **Father:** I am tired. I say what she should have done.
> **Therapist:** And what happens then? **Father:** She says I am never here. Then we do not talk.

What this version does. It slows everything down, asks about inner process rather than behaviour, links the mother's reactivity to her history without interpreting it, and brings the father in as a person with a process rather than as an absent authority. It gives no task in ten minutes and does not intend to.

Version C — solution-focused.

> **Therapist:** I want to ask a different kind of question before we talk about the problem. Suppose tonight, while you are asleep, the school situation is sorted. What is the first thing you notice tomorrow morning that tells you something has changed? **Mother:** (pause) I would wake up and not feel sick.
> **Therapist:** You would wake up and not feel sick. What would you do then, that you do not do now? **Mother:** I would make breakfast. I would call her once, normally, and then I would sit down.
> **Therapist:** Amara, what would your mother see you doing? **Amara:** I would get up.
> **Therapist:** You would get up. When was the last morning that happened, even a bit? **Amara:** September. When my aunty's wedding was on.
> **Therapist:** What was different about that morning?

What this version does. It begins with a preferred future rather than a problem, produces a behavioural description, recruits the adolescent, and finds an exception inside three minutes. Note that it does not deny the problem: the mother's "not feel sick" is a precise description of suffering, elicited without a single question about the problem.

What the comparison teaches. All three versions are recognisably competent. They differ in what they treat as data, in what they do with the first ten minutes, and in what they leave for later. A student who can hear which model is speaking, from the transcript alone, has reached level 1 and is on the way to level 2.

The exercise. Read the three versions aloud in groups, one student per version, without announcing which model. The listeners identify the model and name the two sentences that gave it away. The sentences are almost always the same: the structural version gives itself away with a decision; the Bowen version with a question about inner process; the solution-focused version with a question about a future.

1.9 Laboratory: building the matrix

The week's practical runs in class and continues for six weeks. It is worth 20% of the course, and it is the single most useful document a student will produce this term.

The specification. A spreadsheet or table with nine rows (the nine schools in this course) and eight columns (the six required dimensions plus cultural assumptions and blind spot). One to three sentences per cell. Every cell must be in the student's own words: copied text is not accepted, and the plagiarism check will find it.

The build schedule. One row per week, completed before the seminar; the row for a model is discussed in the seminar and revised afterwards. By Week 7, nine rows are complete and the matrix is submitted with a covering note.

The covering note (500 words). Three questions: which model was hardest to fill and why; which cell did you change your mind about during the term; and what does the matrix as a whole tell you about the field that you did not know in Week 1?

Marking rubric.

Criterion	Weight	Full marks
Completeness	20%	Nine models × eight dimensions, no empty cells, no copied text
Fidelity to each model	30%	The theory of problem of each model is stated in that model's own terms, not reduced to another's
Derivative coherence	20%	Techniques and therapist position are visibly derived from each model's theory of problem
Evidence accuracy	15%	The evidence cell distinguishes what is strong, moderate and thin, and does not overstate
Critical columns	15%	Cultural assumptions and blind spots are specific, not generic ("all models have biases")

The two commonest failures. The first is a matrix in which every model's theory of problem is a paraphrase of the same idea — "the problem is maintained by the family's patterns" appears nine times, which tells the marker that no model has been understood. The second is an evidence column that says "limited evidence" for every model, which is true of none of them and useless in all.

A worked cell, to show the standard. For Milan systemic therapy under theory of problem: "Problems are maintained by the family's belief system and by the punctuation of events that the family and its helpers share; the symptom is a communication in a system that has become rigid around a premise nobody has examined." That cell is specific, it is in the model's terms, and it could not be transplanted into another model's row.

1.9a An annotated matrix row, and a row that fails

Two rows for Milan systemic therapy: one that would earn full marks, one that would earn a bare pass. The comparison is the quickest way to teach the standard.

The row that works.

> Theory of problem: The family's difficulties are maintained by a belief system — a premise held by the family and shared with its helpers — and by the punctuation of events that keeps the premise unexamined; the symptom is a communication in a system that has become rigid around the premise. Unit of focus: The family's meanings, and the observing system that includes the therapy team and the referring agency. Theory of change: The premise must be made visible and disturbed; change follows from new information and difference, not from instruction. Therapist position: Curious, neutral (in the Milan sense of neutral toward all descriptions), working with a team behind a screen or, in later versions, with a reflecting team. Signature techniques: Circular questioning, positive connotation, rituals, the hypothesising-team intervention, the letter. Evidence base: Thin as a model in randomised trials; the components — circular questioning, reflecting teams, systemic hypothesising — are widely used and studied qualitatively; there is no strong specific evidence for Milan systemic therapy as a package with any presentation, and the honest cell says so.

Why it works. The theory of problem is in Milan's own vocabulary ("premise", "punctuation", "communication"), the theory of change is derived from it rather than

added, the therapist position includes the team because the team is constitutive of the model, and the evidence cell distinguishes the package from the components and declines to inflate either.

The row that fails.

> Theory of problem: Family problems come from bad communication. Unit of focus: The family. Theory of change: Better communication. Therapist position: Neutral. Signature techniques: Circular questions, reframing. Evidence base: Limited.

Why it fails, cell by cell. "Bad communication" could describe five models on this course and therefore describes none. "The family" is the unit of focus for every model in the book. "Better communication" is a restatement of the problem, not a theory of change. "Neutral" is a word Milan borrowed and redefined; the row does not say what the redefinition is. "Reframing" is not a Milan technique in the technical sense. And "limited" in the evidence cell is the flat answer that tells the marker the student has not read anything.

The lesson, stated once. A matrix row is good when a reader who knows the model could identify it from the row alone, with the model's name removed. Try that test on your own rows each week: cover the name and see whether the row is identifiable. If it could be any of three models, rewrite it.

1.10 Six errors in comparing models

1. **The catalogue.** Learning nine lists of techniques and no theories. Fix: derive the techniques from the theory of problem for every model, out loud, before you write the row.
2. **The reduction.** "They are all doing the same thing with different words." Sometimes a useful hypothesis and always an untested one. Fix: the reduction test in 1.7, applied to the model's distinctive claim.
3. **The straw model.** Comparing the strongest version of your own model with the weakest version of another's. Fix: read one primary text per model, not only the summary in a textbook.
4. **The flat evidence column.** Treating all models as equally evidenced or equally unevidenced. Fix: distinguish between specific evidence (a model tested for a presentation) and component evidence (an element used across models), and say which you mean.
5. **The cultural silence.** Filling eight columns and leaving out the assumptions that make the model possible. Fix: for each model, ask what kind of family its picture of health describes, and who that picture excludes.
6. **The premature commitment.** Choosing a favourite model in Week 2 and reading everything thereafter as support for it. Fix: the essay requires you to argue against your own preferred model; do that draft early.

1.10a The viva question bank

The viva in Week 15 examines the matrix, the case study and the position paper. These are the questions it is built from, and they can be practised in pairs from Week 1.

On comparison. "Compare two models' theories of change and say which you would choose for this family, and why." "Which two models on your matrix have the most similar theory of problem, and what follows from the similarity?" "Which cell in your matrix was hardest to fill, and what does that tell you about the model?"

On fidelity. "Demonstrate, in words, how a Milan therapist would respond to this sentence." "What would a solution-focused therapist do differently in the first five minutes, and why?" "Which sentence in this transcript is not consistent with the model, and what would the model's author have said instead?"

On evidence. "What is the strongest evidence for this model, and with which presentation?" "What would you say to a family who asked whether this approach works?" "Where is the evidence thinnest, and what do you do about that in practice?"

On critique. "What is this model's blind spot?" "What cultural assumptions does this model carry, and how would they land with this family?" "What would have to happen in a session for you to conclude that the model was the wrong choice?"

On integration. "Combine two models for this case and justify the combination, the sequence and the point at which you would switch." "What would incoherence look like in your plan, and how would you avoid it?"

On self. "Which model are you most tempted by, and what does that temptation tell you?" "What are you not yet able to do at level 2?" "What will you have changed your mind about by the end of the course?"

How to practise. Take one question per seminar, answer it in ninety seconds without notes, and let a peer ask the follow-up. The viva is twenty minutes of exactly this, and the students who do it weekly are unrecognisable by Week 12.

1.11 Culture, faith and the comparative gaze

Comparison is not culturally neutral. Three cautions.

The models were made in particular places. Structural therapy in the Philadelphia of the 1960s, Milan in the Italian psychiatric reform, solution-focused therapy in Milwaukee, narrative therapy in Adelaide and Auckland, emotion-focused therapy in Ottawa. Each carries the assumptions of its setting, including assumptions about authority, individuality, the state, the family form, and the therapist's mandate. A model exported without those assumptions examined will misread families, and will do so most confidently where the family is least like the model's origin.

The comparative gaze can become a ranking. A matrix invites the question "which is best?", and the answer will always be shaped by the culture of the assessor. A model that prizes individuation will rank a model that prizes hierarchy as backward; a model that prizes hierarchy will rank the first as naive. The professional discipline is to hold the matrix as descriptive, and to answer "best for whom, for what, in whose judgement?"

Faith enters at the level of the model's anthropology. Every model carries an implicit account of what a person is: a system of internal objects, a differentiated self, a member of a structure, a storyteller, an attachment-seeking organism. Families whose faith gives them a different anthropology — a person as a soul under authority, as a member of a covenant community — will experience some models as congruent and others as alien. The comparative question is therefore also a theological one, and it belongs in the matrix: which model's picture of a person can this family recognise?

> **DEBATE**
> "Any attempt to compare models on a single grid will favour the models that fit the grid's assumptions." Argue for and against, using one model from this course that resists the six dimensions.

1.11a Where each model's theory of problem came from

A short historical note for each of the nine schools, because a model's origin explains its blind spot more reliably than its textbook description does.

Model	Origin and founding problem	What the origin explains about the blind spot
Psychoanalytic family therapy	Post-war Britain; the problem was the internal world carried into relationships	Attends to depth; under-attends to structure, money and institutions
Bowen	Post-war America; the problem was the transmission of anxiety across generations	Attends to the multigenerational system; treats the present context thinly
Structural	Philadelphia, 1960s; the problem was the organisation of poor, multi-problem families	Attends to hierarchy and action; can miss meaning, loss and culture
Strategic / MRI	Palo Alto, 1960s; the problem was the attempted solution that maintains the symptom	Attends to sequence and to what works; can be manipulative and under-attends to consent
Milan	Italy, 1970s; the problem was the family's premises and the helpers' punctuation	Attends to belief and to the observing team; can be intellectual and slow
Experiential	America, 1960s–70s; the problem was the absence of genuine contact and feeling	Attends to immediacy and the therapist's use of self; thin on evidence and structure
Behavioural / CBT couple	America and Britain, 1970s; the problem was the reinforcement of aversive exchange	Attends to observable behaviour; can under-attend to meaning and attachment
EFT	Ottawa, 1980s; the problem was the rigid cycle organised around unmet attachment need	Attends to emotion and attachment; may under-attend to material conditions
Solution-focused	Milwaukee, 1980s; the problem was the problem-saturated description	Attends to solutions; can bypass suffering and history
Narrative	Adelaide and Auckland, 1980s–90s; the problem was the internalisation of dominant stories	Attends to meaning and power; can remain at the level of description

How to use the table. In the viva, the question "what is this model's blind spot?" is answered best by pointing to the problem the model was built to solve. Every model is a

solution to a historical problem, and it inherits the blindness of that problem. A structural therapist in 1960s Philadelphia was not asked to think about colonial internalisation; a narrative therapist in 1990s Auckland was not asked to manage a psychosis relapse. The model is not at fault; the therapist who applies it outside its problem is.

1.12 Evidence note: what comparison can and cannot establish

The comparative literature is large and its findings are less exciting than students expect.

What is established. Bona fide models produce broadly similar outcomes across many presentations; differences, where they appear, are usually small and often confined to specific presentations or sub-groups. The strongest specific effects are where a model targets a well-defined mechanism in a well-defined condition — family-based treatment for adolescent anorexia is the standard example. Therapist effects are substantial, and the alliance is among the more robust predictors across models.

What is not established. That any one of the nine schools is superior overall; that model allegiance predicts outcome; that training in more models produces better outcomes. The comparative question "which model is best?" has no general answer in the literature, and clinicians who claim one are usually reporting their own experience.

What follows for the matrix. The evidence column should be filled with three questions in mind: what is the evidence for this model as a whole; what is the evidence for this model with a specific presentation; and what is the evidence for the components the model uses. A model can be weakly evidenced overall and strongly evidenced for one presentation — this is common — and a matrix that records only the first number will mislead the student who relies on it in a viva.

The professional claim. "I am using this model with this family because it targets the mechanism I believe is maintaining the problem, because there is evidence for it with this presentation, and because it is within my competence — and I will measure whether it is working." That sentence is defensible in any of the three assessments, and it is the sentence this course is designed to produce.

1.13 Key terms

Term	Definition
Theory of problem	A model's account of how difficulties develop and what maintains them
Unit of focus	What the model treats as the system of interest (individual, dyad, structure, meaning, context)
Theory of change	A model's account of what must happen, and in what order, for improvement
Therapist position	The role the model requires of the therapist, and the authority it confers
Signature technique	A characteristic method that follows from the model's theory of problem
Evidence base	The strength, quality and specificity of research support for a model
Blind spot	What a model systematically fails to notice, usually the shadow of its strength
Reduction test	Whether a proposed simplification preserves the model's distinctive claim

Term	Definition
Charity test	Stating an objection in its strongest form before answering it
Level 1 / 2 / 3	Describe, demonstrate, critique — three separable competences with a model
Model fidelity	The degree to which a session is recognisably conducted in the model's terms
Component evidence	Evidence for an element used across models, as distinct from evidence for a whole model

1.13a Two student matrices, and what the marker sees

Two submissions, both technically complete, with very different marks. The comparison is worth an hour of seminar time.

Submission A. Nine rows, all cells filled, all in the student's own words, each model described in its own vocabulary. The evidence column distinguishes model-level from component evidence. The blind-spot column names specific consequences: "structural work with a family whose grandmother holds executive authority will read her authority as a boundary violation and will fail at the point of implementation". The cultural column is specific: "assumes a nuclear household with two parents available for enactment".

Submission B. Nine rows, all cells filled, all in generic language. "Theory of problem: dysfunctional family patterns. Unit: the family system. Theory of change: restructure the system. Position: active. Techniques: enactment, reframing, questions. Evidence: mixed." The blind-spot column says "may not suit all families". The cultural column says "must be culturally sensitive".

What the marker sees. Submission A demonstrates that nine models have been read and differentiated; submission B demonstrates that nine models have been noticed. The difference is visible in three specific places, and students should check their own drafts against them.

First, the vocabulary test. Does the row use words that belong to the model, or words that belong to the field? "Bad communication" belongs to the field; "punctuation" belongs to Milan; "the attempted solution" belongs to the strategic school; "the cycle" belongs to EFT. A matrix written in the field's vocabulary cannot distinguish the models, because the field's vocabulary is the lowest common denominator of all of them.

Second, the consequence test. Does the blind-spot column name a consequence, or a sentiment? "May not suit all families" is a sentiment. "Will fail at implementation when the family's authority structure is not the one the model assumes" is a consequence, and it tells the reader what to watch for.

Third, the honesty test. Does the evidence column record what is actually known, including the awkward parts — that Bowen theory has little trial evidence, that solution-focused therapy has better evidence for its components than for its package, that EFT's evidence is strongest for couple distress and thinner for other presentations? A matrix that is uniformly cautious is as uninformative as one that is uniformly enthusiastic.

1.14 Self-test

1. Name the six dimensions of comparison, and the two the course adds.
2. Why is the theory of problem described as the master dimension?
3. Derive the therapist position and two techniques for the Bowen model from its theory of problem.
4. What are the three levels of knowing a model, and which assessment tests each?
5. What is the reduction test, and what does it protect against?
6. What is the charity test, and why does the essay require it?
7. What three questions should the evidence column answer?
8. Give one cultural assumption carried by each of the structural and solution-focused models.

Answers. 1. Theory of problem; unit of focus; theory of change; therapist position; signature techniques; evidence base — plus cultural assumptions and the characteristic blind spot. 2. Because the account of what maintains a problem determines what the model attends to, what it does, what role the therapist takes, and what counts as change; the other five dimensions can be derived from it. 3. Position: coach, neutral observer of process. Techniques: the genogram and coaching the parents to lower their own reactivity (detriangling questions). 4. Describe (written examination), demonstrate (observed session), critique (essay and viva). 5. A test of whether a proposed simplification preserves the model's distinctive claim; it protects against dismissal disguised as summary. 6. Stating an objection in its strongest form before answering it; the essay requires it because a model defended against a weak objection has not been defended. 7. What is the evidence for the model as a whole; for this model with this presentation; and for the components the model uses. 8. Structural: that a family should have a clear hierarchy with parents in charge. Solution-focused: that a family can describe a preferred future and act on it.

1.15 Reflective task

Before the seminar, write 300 words on the following: which of the nine models in this course do you already believe, and why do you believe it? Most students discover that they hold a model without having chosen it — usually the one taught by the most persuasive lecturer, or the one that fitted their own family. Name it, state the evidence you actually have for it, and state one thing the model cannot see. Bring the paragraph; it will be revised in Week 14.

1.16 Readings for this week

- **Nichols & Davis**, chapter 1, for the survey of the field and the standard comparison frame.
- **Lebow (2014)**, Couple and Family Therapy: An Integrative Map of the Territory, chapters 1–2, for a systematic comparison that treats the models as answers to clinical questions.

- **Gurman & Kniskern** or **Sprenkle, Davis & Lebow (2009)**, for the common-factors argument that will challenge the matrix you are building.
- **One primary text for a model you think you already know**, read for an hour, to test whether you know it at level 1 or only at the level of the summary.
- **Carr (2019)**, the opening chapters, for the history and the structure of the field.
- **The course plan on the previous page**, read against this week's grid: for each of the nine weeks, write the one question you expect that model to ask, and bring the nine questions to the seminar. The exercise takes ten minutes and gives you a set of hypotheses to test against the reading, which is a much better use of the first week than trying to remember nine definitions you have not yet met.

Next: Week 2 — Psychoanalytic and object-relations family therapy

---

Week 2 — Psychoanalytic and object-relations family therapy

The question it asks. What unconscious processes, carried from earlier relationships, are being re-enacted in this family now?

2.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Why start here	The first family therapists were psychoanalysts, and their problem is still live	Name one thing in a family that only this model explains well
10–25	Object relations in plain language	Internal working models, with two examples from the case bank	Map one family's internal objects
25–40	Projection and projective identification	The mechanism, demonstrated on a transcript	Locate the projection in a recorded session
40–55	Transference and countertransference in family work	Why the therapist's feeling is data	Identify their own response to a role-play
55–70	Interpretation, used sparingly	Four rules for an interpretation that will land	Draft and test one interpretation
70–85	The Tavistock and group-analytic traditions	Work with the family as an unconscious system	Compare with the individual model
85–90	Critique and the matrix row	What the model cannot see	Fill the row

2.2 Core concepts

The model's vocabulary is old and its mechanisms are still in use across the field, including by therapists who believe they do not use them. Learn the five concepts precisely.

Object relations. The idea that a person's early relationships are internalised as working models of self and other — a "mother who cannot be relied on", a "self who must not need anyone" — and that these internal models shape how later relationships are perceived and conducted. Family work extends the idea: each member brings a set of internal objects into the household, and the family's relationships are a negotiation between them.

Projection. Disowning a feeling in oneself and locating it in another person. A father who cannot tolerate his own fearfulness experiences his daughter as fragile and protects her excessively. The mechanism is not deception; it is usually unconscious, and it is why a family's description of one member often tells you more about the describer than the described.

Projective identification. Projection plus pressure: not only is a disowned part located in the other, but the other is recruited, often subtly, to behave in accordance with it. The mother who cannot bear her own helplessness treats her son as helpless, organises his day, speaks for him, and in time he presents as helpless — and both are sincere. This is the model's central explanatory mechanism for the identified patient, and it is the mechanism most often misapplied, because it is used to explain away the family's account.

Splitting. Experiencing people as all-good or all-bad, with the two views held separately rather than integrated. In families it appears as the alignment in which one parent is idealised and the other denigrated, or in which one child is the good one and another the bad one. The splitting is maintained by the family's anxiety: a whole object is harder to hold than two halves.

Repetition compulsion. The re-enactment of earlier relational patterns in new relationships, including with the therapist and the therapy. A family that has dismissed three therapists will dismiss a fourth, and the repetition is information.

The family as a shared unconscious system. The synthesising idea of the school: the family is not a collection of individual psyches but a system with its own unconscious assumptions, shared myths, and defended-against feelings, and one member often carries the feeling the system cannot hold. This is the clearest statement of the model's theory of problem, and it is what distinguishes the school from a set of individual psychodynamic sessions conducted in the same room.

2.2a Four concepts, one family

The concepts become learnable when they are applied to a household the student can picture. The Adeyemis, read object-relations.

The internal objects each partner brought. The mother's father died when she was 19 and she raised four siblings while her own mother was unwell. Her internal model of a reliable other is a person who nonetheless cannot be leaned on, and her model of herself is a person who copes. The father's mother died when he was 16 and he left for Lagos that year. His internal model of loss is that the way to survive it is to leave, and his model of himself is a provider who is not needed at home. Neither model is a pathology; both are adaptations that made sense in an earlier circumstance.

The projection. The mother experiences her own unmanageable grief and helplessness — about her father, about her own mother, about the years of coping alone — and locates it in Amara, who is described as fragile, unable to cope, and needing to be at home. The language the mother uses about her daughter ("she cannot face it", "she is not strong enough") is language a person might use about themselves.

The projective identification. The mother's care does not merely describe fragility; it organises the morning so that Amara is never required to manage anything. Amara is spoken for, protected from the school's letters, and relieved of the consequences. In time, Amara behaves as described — staying in her room, unable to leave — and both mother and daughter are entirely sincere.

The splitting. The family's account divides the adults cleanly. The mother holds the practical and the emotional labour and is seen as strong; the father is seen as absent and uncomprehending. Each holds one half of a whole picture, and neither can see that the mother's strength is a defence and the father's absence is a grief.

The repetition. The mother is 19 again, in effect, managing a household's feeling for other people. The father is 16 again, leaving a house in which someone has been lost. The repetition is not a curse; it is the human tendency to return to the pattern that once worked, especially under stress.

What the formulation gives and what it costs. It gives a compassionate account of both parents and a mechanism for the daughter's presentation that explains why behavioural plans have failed. It costs attention to the school's timetable, the family's money, the father's employment, and the possibility that Amara's refusal is a reasonable response to something nobody has asked about. Both halves of that sentence belong in the formulation.

2.3 Therapy: the shape and the mechanism

The shape. Longer-term than most of the models in this course, less structured, and less task-oriented. Sessions are typically weekly and open-ended; the therapist's attention is on what is happening between the members and between the family and the therapist; the work moves slowly and is expected to.

The mechanism. Two elements, both contested and both useful.

First, containment. The therapist holds difficult feelings without evacuating them — that is, without immediately turning them into advice, reassurance or a task. The family that has never had a feeling held will discover, in the holding, that the feeling can be survived. The mechanism is experiential rather than interpretive, and it is the part of the model that transfers most readily to other approaches.

Second, interpretation. The therapist offers an account of the pattern, including its unconscious dimensions, in a form the family can hear. Interpretation is not insight delivered; it is a hypothesis offered tentatively, and it works when the family's response is recognition rather than explanation.

Transference and countertransference. The family's feelings towards the therapist and the therapist's towards the family are treated as data about the family's relational world. A therapist who feels useless with a family should ask what the family does with people who try to help — a question that is far more informative than the therapist's self-criticism. A therapist who feels protective of one member should ask who else in the system feels that way, and who does not.

Why this matters even to students who will not practise this way. Every model in this course uses the therapist's response as information. This school articulated the principle first, and it is the reason supervision in family therapy attends so closely to the therapist's feelings.

2.3a How long is this therapy, and who can have it?

The model's length is not incidental; it is part of its theory of change, and a student who cannot discuss it cannot defend the matrix row.

The classical claim. Change of the kind this model seeks — a shift in internal models, in what the family can bear to feel, in the therapist's and family's capacity to stay with difficulty — is slow. The school's own logic implies months rather than weeks, and its founders generally wrote about treatment lasting a year or more.

What the modern practitioner does with that claim. Three options, each defensible.

Option 1 — practise it as designed, in a service that can fund it. Specialist psychotherapy services exist and are justifiable. The professional obligation is to be honest about the cost and the access, and to measure outcomes rather than assume them.

Option 2 — use the model's mechanisms in a shorter therapy. Containment, the therapist's response as data, the observation of what is not said, and one well-timed interpretation can all be used within a twelve-session contract. This is what most graduates will actually do, and it is not a betrayal of the model; it is the model's components deployed without its frame.

Option 3 — use it as a second phase. Some families need a structural or behavioural first phase (see MFT 701 Week 13) and can use a psychodynamic phase afterwards, when the presenting crisis is settled and the family has the resources for slower work. This is the commonest integration in practice, and it is worth being able to describe.

What none of the three options permits. Practising a slow, open-ended therapy with a family who cannot afford it, cannot reach the clinic, or needs a crisis resolved this week, and calling the mismatch "resistance". The model's silence about access is a limitation, not a licence.

The question for the viva. "You have twelve sessions with this family. What would you keep from this model and what would you drop?" A strong answer keeps the mechanisms and drops the length, names the risk (that the family experiences a short therapy as another person who did not stay), and says how it would be measured.

2.4 Signature technique: interpretation, used sparingly

An interpretation is a statement that links present behaviour to an unconscious process, offered tentatively, in the family's language, at a moment when it can be heard. Four rules separate an interpretation from an accusation.

Rule 1 — link, do not label. "When your mother talks about her own mother, you answer for her" is a link between an event and a behaviour. "You are enmeshed" is a label. Links can be tested; labels have to be accepted or refused.

Rule 2 — offer, do not pronounce. "I wonder whether..." and "I notice... could it be that..." are the grammar of interpretation. A statement delivered as fact invites compliance or defiance, and the model's own theory says the family's response to a pronouncement is also the family's work.

Rule 3 — interpret the pattern, not the person. "Something seems to happen in this room when your father begins to speak about his own father" is a statement about the system. "You are protective of your father" is a statement about a person, and it recruits the person's defence.

Rule 4 — interpret one step beyond what the family already knows. An interpretation the family has already made is tedious; one that is two steps away is a lecture. The therapist's judgement is about the size of the step, and the family's response is the evidence.

The worked example from the course material. "I notice that whenever your mother speaks about her own mother, you begin to speak for her. I wonder whether that is how the family has always managed her sadness." The interpretation has four parts: an observation (the sequence), a link (speaking for her), a hypothesis (the family's way of managing a feeling), and a tentative frame ("I wonder"). The son's answer — whether "yes, someone has to" or "no, she loses her words" — will be more informative than the interpretation itself.

What to do when the interpretation is rejected. Say "that may be wrong", and stay curious. A rejected interpretation is not a failure; it is data about the family's defences and about the therapist's timing. The model's own logic requires the therapist to survive the rejection without withdrawing, because the survival is itself the therapeutic act.

2.5 Contribution, and what the field kept

The school's contribution is larger than its current evidence base, and students should be able to say what has been absorbed into the wider field.

Kept: the respect for what is not said. Every model now attends to what a family cannot speak about. The psychoanalytic school made this a principle rather than an accident.

Kept: the therapist's emotional response as data. Now standard across models, and traceable to this one.

Kept: the idea that a symptom can carry a feeling the family cannot hold. This is the ancestor of the Milan positive connotation, the structural identification of the identified patient, and the systemic idea of function.

Kept: the family as a shared unconscious system. The notion of a family myth, a family secret and a shared assumption is now ordinary across the field, and it began here.

Kept: containment as a mechanism. Whatever the model, the therapist who can stay with a family's distress without converting it into a task is doing something this school described first.

What the field discarded, and why. Long open-ended treatment as a default; the assumption that insight is necessary for change; the pathologising vocabulary ("the intrusive mother", "the schizophrenogenic mother"); the indifference to material context; and the absence of outcome measurement. Each discard is a lesson in what happens to a model that does not examine its own assumptions.

2.5a The Tavistock tradition and group-analytic family therapy

Two institutional traditions shaped this school, and a student who cannot place them cannot read the literature.

The Tavistock. The Tavistock Clinic and the Tavistock Institute in London produced both the group-relations tradition and a substantial body of family therapy. The distinctive features are the attention to unconscious group process, the use of a work-group and basic-assumption frame, and — clinically — an interest in the family's shared assumptions and in the therapist's use of their own reaction. Tavistock-trained family therapists typically work in long-term, once-weekly formats, often with co-therapists, and often with an observing team.

Group-analytic family therapy. Derived from Foulkes's group analysis, this approach treats the family as a small group with a group matrix — a shared field of communication and meaning — and works with the group's dynamics as much as with individual members. The therapist's task is to facilitate the group's own capacity to think, rather than to interpret individual behaviour. The vocabulary is different from the classical school and the practice is often shorter and more active.

What the two traditions contribute to the matrix cell. Both extend the unit of focus: not the individual internal world alone, but the family's shared unconscious field. Both make the therapist's membership of the system explicit. Both raise the question of the observing team, which Milan later developed into a technique.

What they share with the classical school. The assumption that what is not said is the most important material; the attention to the therapist's emotional response; and a long time frame.

What a student should take from this section. Two or three named traditions, one distinguishing feature of each, and one sentence on how they differ from the classical model. That is sufficient for level 1, and it is more than most candidates can offer.

2.6 Blind spots, stated precisely

Blind spot 1 — the vocabulary blames parents. The classical language locates pathology in maternal behaviour, and the history of the school includes theories that caused real harm to mothers of children with psychosis. The modern practitioner can retain the mechanisms and must discard the blaming grammar; the test is whether the family experiences the interpretation as an accusation.

Blind spot 2 — thin evidence in its classical form. There is little randomised evidence for long-term psychoanalytic family therapy as a package. There is better evidence for psychodynamic psychotherapy in individual form for some presentations, and there is evidence for components — mentalisation, reflective functioning, containment — that the school anticipated. The matrix cell should record this distinction.

Blind spot 3 — cost, time and access. An open-ended weekly therapy is unavailable to most families in most services. A model that requires conditions most families cannot have is not wrong; it is limited, and the limit belongs in the matrix.

Blind spot 4 — the material and the institutional. A formulation that locates the difficulty in the family's internal world can miss a landlord, an employer, an immigration status or a school. The model's unit of focus is the internal and the relational; the external is under-described.

Blind spot 5 — the therapist's authority is under-examined. The interpretive position is a powerful one, and the model has historically been less attentive than the Milan or narrative schools to the power the interpreter holds.

> **CASE**
> A young man cannot keep a job; his mother calls his employers before he does. Object-relations reading: the mother's own abandonment anxiety is projected into her son's incompetence, and his failure keeps her needed. Interpretation may be less useful than an experiment: agree that she will not call, and predict that both will feel anxious for two weeks. The prediction turns insight into something testable.

2.6a What the school got wrong, in its own history

A professional course owes students the history of a model's errors, because the errors are the clearest guide to where a mechanism can be misapplied.

The schizophrenogenic mother. From the 1940s to the 1970s, a body of theory held that schizophrenia in a young person was caused by a particular kind of mother — cold, domineering, covertly rejecting. The idea was influential, was taught in good faith, and caused enormous harm: mothers were blamed for a condition now understood as neurodevelopmental, families were divided by the accusation, and a generation of clinicians was trained to see maternal care as the cause of serious mental illness.

Why the idea was plausible. It fitted the model's mechanism (the family as an unconscious system; the symptom as the container of an unheld feeling), it was supported by clinical observation of distressed families, and it explained the observations that others

could not — that these families often did look unusual. What it lacked was any consideration of direction of causation: families in which a young person is psychotic look unusual because they are living with psychosis.

What the field learned, and what it cost to learn it. Three lessons, all now standard.

First, observation of a distressed family cannot establish causation. The family's state may be a response to the difficulty rather than its source. This is the single most important methodological caution in family therapy, and it applies to every model in this course, not only this one.

Second, a mechanism that explains everything explains nothing. The concept of the family as a shared unconscious system is powerful precisely because it can be applied to any family, which means it cannot discriminate between them without additional constraints.

Third, the vocabulary of a model has consequences for families. "The intrusive mother" was not a neutral description; it was a diagnosis delivered to a person who was already struggling, and it shaped what services offered and what families believed about themselves.

How a modern practitioner uses the school despite its history. By keeping the mechanisms — projection, projective identification, containment, the therapist's response as data — while refusing the vocabulary of blame, and by asking, of every interpretation, whether the family would experience it as an accusation. That test is the professional inheritance of this history, and it should be applied to the models in Weeks 3 to 11 as rigorously as to this one.

2.7 Annotated transcript: an interpretation that lands, and one that does not

The same family, the Okonkwos, is seen by the same therapist in two sequences. The first interpretation is heard; the second is not, and the difference is instructive.

Sequence A — session three.

> **T1 Therapist:** Can I say something I have noticed, and you will tell me whether it is right? When your wife talks about Ebuka's mornings, you look at the window. When she stops, you look back.
> **T2 Father:** (pause) I did not know I did that.
> **T3 Therapist:** I wonder whether the window is where you go when the conversation is about something you cannot fix from Lagos.
> **T4 Father:** (long pause) It is. She tells me and I cannot do anything, so I stop hearing it.
> **T5 Therapist:** And then she has told you and you have not heard it, and she is alone with it. That might be the thing that is hardest in this house.
> **T6 Mother:** (quietly) Yes.

Annotations. T1 is an observation, offered with an explicit invitation to correct it. The observation is specific and behavioural — looking at the window — which is what allows the father to recognise it rather than dispute it. Note that the therapist does not begin with the concept.

T3 is the interpretation, and it is one step beyond what the father has said. It links the behaviour to a feeling (helplessness) and to a circumstance (Lagos), which keeps the interpretation attached to reality rather than to the father's character. The grammar is tentative.

T4 is the moment the interpretation lands, and the marker of landing is not agreement but new information: the father describes a process the therapist had only hypothesised.

T5 does the systemic work. The interpretation has been about one man's behaviour; T5 places it back in the loop, naming the consequence for the mother without blaming the father. This is the discipline that distinguishes object-relations family work from individual interpretation conducted in company.

Sequence B — the same therapist, one week later, with the same family.

> **T7 Therapist:** I have been thinking about last week, and I think I understand what is happening. Ebuka is carrying your sadness about his grandmother, because neither of you has been able to grieve, and children always carry what the parents cannot.
> **T8 Mother:** (silence)
> **T9 Father:** He is nine. He does not know anything about grieving.
> **T10 Therapist:** I did not mean that he understands it. I meant that he feels it.
> **T11 Father:** (to his wife) This is why I did not want to come.

Annotations, and what went wrong. T7 breaks three of the four rules. It is a pronouncement ("I think I understand what is happening"), not an offer; it is two steps beyond what the family knows, not one; and it interprets a person — Ebuka — rather than a pattern, in Ebuka's absence. The sentence "children always carry what the parents cannot" is a general law delivered to a family, and general laws invite families to defend themselves against the therapist's theory.

T9 is not resistance; it is a reasonable objection to an interpretation that has not been earned. The father's defence is accurate: the therapist has claimed knowledge of his son that the therapist does not have.

T10 repeats the interpretation in other words, which is the commonest response to a failed interpretation and the least useful. T11 is the cost: the father, whose engagement cost the therapist three sessions of careful work, withdraws.

The repair, done properly at the start of the next session.

> **T12 Therapist:** Last week I said something about Ebuka carrying your sadness, and it did not fit. I have been thinking about why. I think I was talking about my idea rather than about what either of you had told me. I would like to go back to what your husband said — that Ebuka is nine and does not know about grieving — because I think he was right, and I want to understand what he meant.

Why the repair works. It names the error without self-abasement, credits the person who objected, returns the authority to the family, and asks a genuine question. In this

model, the therapist's survival of an error — and the family's experience of a therapist who can be corrected — is itself the therapeutic mechanism.

2.7a What the therapist brings: the use of self in a slow model

In this model more than most, the therapist's own history is an instrument, and an unexamined instrument is a hazard.

The claim. The therapist's emotional response is data (2.2), and it is also contamination if it is not examined. A therapist who was parentified as a child will feel a strong pull to protect a parentified adolescent, and the pull will arrive before the thought. The model's answer is not to eliminate the pull but to know it, which requires the therapist to have done some of the work the model asks of families.

Three disciplines, in order of importance.

First, know your own object-relations pattern. The genogram of MFT 701 and the cultural genogram are the beginning; personal therapy, where it is available and appropriate, is the standard the tradition itself assumes. A therapist who cannot name their own central relational pattern will enact it with families, reliably and invisibly.

Second, use supervision as the place where the countertransference is read. The countertransference log of 2.8a is the instrument; supervision is where the fourth column is interrogated. A supervisor's most useful question is often "and whose feeling do you think that was?"

Third, notice the families you dread. The family you avoid booking, the one you feel relief about when they cancel, the one you describe in the team meeting with a sigh — each is information about your own pattern, and each is a training opportunity rather than a personal failing.

The boundary of the discipline. Using the self is not the same as disclosing the self. A therapist's decision to share something personal should be a clinical decision with a rationale, not an expression of the countertransference. The model's own tradition is cautious here, and rightly: the family came for its own work.

The protection. Where a therapist's own history is activated repeatedly and strongly by a family, the professional response is supervision, a reduction in the caseload if possible, and personal therapy taken seriously as part of the cost of doing this work rather than as a personal indulgence. This model, of all the models in this course, requires the therapist to be a person who has been attended to, and the requirement is not a luxury; it is a competence.

2.8 Laboratory: finding the projection

A ninety-minute practical in three parts.

Part 1 (20 minutes) — the transcript. Students receive a four-page transcript of a first session with a family in which a 14-year-old is described by both parents as "the angry one". The transcript contains eleven instances of one mechanism: the parents describe their daughter's anger in language that describes their own unexpressed anger ("she is

always simmering", "she cannot let anything go", "she explodes over nothing"). Students mark every instance and write the sentence the therapist could say.

Part 2 (40 minutes) — the role-play. Triads: two parents and a therapist. The parents are given a brief: each is carrying a disowned feeling (one shame, one anger), and neither knows the other's brief. The therapist's task is to find the projection without naming it as a projection, using observation, tentative linking and the family's own language. The observers score against the rubric.

Part 3 (30 minutes) — the debrief. Each therapist reports what they felt towards each parent, and the group maps those feelings onto the briefs. The exercise usually demonstrates the model's central claim: the therapist's countertransference arrived before the therapist's understanding.

Marking rubric.

Criterion	Weight	Full marks
Observation before interpretation	25%	At least three specific behavioural observations quoted before any interpretation
Tentative form	20%	Interpretations offered as hypotheses, with the family's correction invited
Pattern, not person	20%	Statements about sequences and the system rather than about a member's character
One-step calibration	20%	The interpretation is one step beyond the family's own account, not two
Survival of rejection	15%	The therapist stays curious after a rejection, and does not repeat the interpretation in other words

The commonest failure. Students interpret in the first ten minutes, before they have observed anything. The marker's note is always the same: an interpretation without an observation is a theory about the family, and families can tell the difference.

2.8a The countertransference log

The model's most transferable instrument is a single page, kept after every session. Four columns and a line.

Column	What to record
What I felt	The dominant feeling in the session, with the moment it began
What I wanted to do	The impulse — to reassure, to rescue, to withdraw, to argue, to finish early
What I did	What actually happened, which is often different
Whose feeling it might be	A hypothesis: mine; one member's; the family's shared feeling; or a response to what I represent

The line at the bottom. "What I will ask about next time."

Why the log works. It converts an unmanageable inner experience into a hypothesis that can be tested in the next session, which is exactly the move the model requires. It takes four minutes, it is the most useful supervision material a trainee can bring, and it protects against the two commonest countertransference errors: acting on the feeling without noticing it, and dismissing the feeling as irrelevant.

An example, from the Okonkwo material. Felt: irritation, then guilt, whenever the father spoke. Wanted to do: interrupt him, and then over-include him. Did: asked the mother a question instead. Whose feeling: possibly the mother's — the irritation at a man who arrives on Friday and comments on the week; possibly mine, from a family in which the father was absent; possibly the family's shared feeling about a father who is not there. Next time: ask the father what it is like to arrive on Friday into a week he has not seen.

The discipline. The fourth column must offer more than one hypothesis. A log in which every feeling is the family's is not a log; it is a way of avoiding one's own history, and supervision will find it out.

2.9 Six errors in object-relations family work

1. **The interpretation without an observation.** A theory about the family delivered before anything has been noticed. Fix: three specific observations, quoted, before any interpretation.
2. **The labelling vocabulary.** "Enmeshed", "intrusive", "over-identified" used as descriptions of a person. Fix: describe the sequence, and let the family supply the adjective.
3. **The interpretation two steps ahead.** Naming a mechanism the family has given no evidence of needing to hear. Fix: one step, and check the response before taking the next.
4. **Interpreting an absent member.** Explaining a child to their parents in the child's absence. Fix: if the person is not in the room, speak about what the parents notice, and invite the person into the next session.
5. **The failed interpretation repeated in other words.** The commonest response to rejection and the least useful. Fix: say "that may be wrong", stay curious, and return to the observation.
6. **The endless therapy.** Continuation because ending has not been planned, justified by the depth of the work. Fix: a stated review point, a theory of what would count as enough, and the discipline to say so to the family before they have to ask.

2.9a Two sessions with the same family, one object-relations and one not

A comparison that shows what the model adds, using the Ogundipe couple from MFT 701's case bank.

The family. Married fourteen years, two children. The wife says the husband is "never really there"; the husband says the wife is "never satisfied". Twice in fourteen years there has been a pushing incident, both when he had been drinking. She has not told her family. He is affectionate with the children and dismissive of his wife's account.

A structural reading. The couple subsystem is weak, the wife is in a coalition with the children, the husband is peripheral, and the wife's escalating demands are an attempt to bring him into the marriage. The first move is to strengthen the couple boundary with a protected weekly conversation and a task that requires coordination.

An object-relations reading. The wife's internal model is of a person who must manage alone, learned in a family in which her mother was depressed and her father absent; she experiences her husband's withdrawal as confirmation that she is unlovable when she needs something, and she escalates in order to extract evidence to the contrary. The husband's internal model is of a person whose needs are dangerous, learned in a family where his own mother's neediness was punished; he experiences his wife's demands as an attack and withdraws. Each is confirmed in the other's least available state. The pushing incidents are not explained by this formulation and must be handled separately and first.

What the object-relations reading adds. Three things. It explains why the escalation is so intense relative to the presenting complaint — the wife is not arguing about the school run, she is arguing about whether she is lovable. It identifies the mechanism by which each partner's behaviour confirms the other's model, which is more explanatory than "the couple subsystem is weak". And it gives the therapist a specific countertransference prediction: the therapist will feel, towards this couple, an impulse to side with whoever seems more abandoned, and will need to notice it.

What it risks. Both formulations are about the couple's internal worlds, and neither mentions the alcohol, the pushing, or the wife's concealment from her family — which are facts about safety and about power, not about internal objects. The safety work comes first in either case, and the object-relations reading must not be allowed to soften it.

The exercise. Write the first three sentences of a session in each of the two models, then write the sentence that would open the session if the therapist had read both. Most students' third sentence is better than either of the first two, which is the case for comparison in one paragraph.

2.10 Culture, faith and the unconscious

The model's cultural assumptions are specific and consequential.

It assumes that the individual internal world is the primary unit. Many families understand a difficulty as arising between people, or between the family and the spiritual world, rather than inside a person. An interpretation offered in the vocabulary of internal objects may be heard as an accusation of individual pathology, and a family whose account is relational or spiritual will find the model's grammar foreign.

It assumes that the therapist's authority to interpret is welcome. In families where the therapist is young, of a different gender or ethnicity, or in a context where professional authority is contested, the interpretive position must be earned rather than assumed. The model's silence on this is a limitation, not permission.

It assumes a particular kind of family boundary. The concepts of enmeshment and intrusion carry a picture of the healthy family as separate individuals in a clear boundary. Families in which three generations share a household, or in which a grandmother holds

executive authority, may be described in the classical vocabulary as pathological when they are simply structured differently.

Care with the spiritual register. A family may experience a difficulty as an attack, a punishment or a test, and may understand the "unconscious" as the spiritual realm. The mechanism of projection can be offered in the family's own register: "it is as though the worry has been placed in him, and now he carries it for the house." That sentence is the model's insight without the model's vocabulary, and it is more likely to be heard.

What the model offers families whose faith is central. A respect for what cannot be said, and for the reality of the unseen. A therapist who can sit with a family's spiritual account without reducing it to psychology is doing something the model requires, even if the school's founders would have described it differently.

> **DEBATE**
> "A theory that locates the problem in the parents' unconscious is a theory that blames parents." Argue for and against, using one case in which the interpretation helped and one in which it harmed.

2.10a Working with a family whose culture does not use the model's vocabulary

An exercise in translation, which is what cross-cultural practice with this model actually requires.

The family's account. A mother describes her son as "carrying something from his father's house", and says the boy "has been like this since the burial". She does not use the word grief, and she does not describe an internal world. She describes a transmissible substance — something carried, something from a house, something that arrived at a date.

The model's account. A disowned feeling has been located in the child and he has been recruited to carry it; the family's shared unconscious has organised around an unmourned loss.

The translation. "It sounds as though something came into this house at the burial and it has been in him since. I want to understand what it is, and who else in the family is carrying it — because I think more than one person is." The sentence preserves the family's own account, uses their vocabulary, and adds the model's central move (that a feeling is distributed in the system and carried by one member) without importing the model's words.

What the therapist must not do. Do not correct the family's account ("what you are describing is grief"), do not adopt it literally and abandon the clinical task ("the burial must have been done improperly"), and do not use the translation as a technique, which families detect. The translation is a way of taking the family's account seriously enough to work inside it.

The follow-up questions, in the family's register. "When he carries it, what happens in the house?" "Who else has carried something like this?" "What would have to happen for it to be put down?" Each question is the model's mechanism asked in the family's language, and each will produce material that a question about "feelings" would not have produced.

The general principle for this week. This model's power is in its attention to what cannot be said. Its danger is in its vocabulary. The therapist who can use the mechanism and speak the family's language has the best of it; the therapist who requires the family to adopt the model's language has mistaken a tool for a creed.

2.11 Evidence note: what is actually known

The matrix cell for this model must be honest, and honesty here means separating three different claims.

Claim 1 — long-term psychoanalytic family therapy as a package. There is very little randomised evidence, and no body of trials comparable to the couple-therapy literature. The evidence that exists is largely case-based, qualitative and drawn from specialist clinics with selected families. The honest cell says: "no adequate trial evidence for the package as delivered classically".

Claim 2 — psychodynamic psychotherapy in individual form. Here the evidence is better: trials and meta-analyses support psychodynamic psychotherapy for depression, some anxiety presentations and personality difficulties, with effects that appear durable after treatment ends. This is evidence for a relative of the model, not for the model.

Claim 3 — the components. Mentalisation-based treatment has trial evidence in borderline personality disorder; reflective functioning is a measured construct with predictive validity; the concept of containment has informed therapeutic work in psychosis. Attachment theory, which grew in adjacent soil, is among the best-evidenced frameworks in the field. A student who can distinguish these three claims can defend the row; a student who says "psychoanalytic family therapy is not evidenced" has said something true and uninformative.

How to use the model responsibly. As a source of mechanisms — containment, the therapist's response as data, the idea that a symptom can carry an unspoken feeling — rather than as a whole treatment to be delivered. Used this way, it is compatible with the evidence-based models in this course, and it is why the school is taught in Week 2 rather than abandoned.

2.12a A reading exercise: three paragraphs of Bion

The model's most difficult concept is containment, and the difficulty is worth meeting directly, because a student who can explain it plainly can explain anything in this course.

The passage. Take two pages of Bion — the account of the infant's fear projected into the mother, who receives it, modifies it, and returns it in bearable form. Read it three times: once for the sense, once for the words you do not know, once for what it would mean in a session.

The plain translation, written in five sentences.

> A person who cannot bear a feeling does not simply hide it; they put it into somebody else. If the other person can take it without being destroyed by it — can feel it, think about it, and give it back in a smaller form — the first person gradually learns that feelings can be survived. If the other person cannot, the feeling comes back unmodified, or the other person becomes ill with it. The therapist's job, in this model, is to be the second kind of person. That is why the therapist's own support matters: you cannot hold a feeling you have no room for.

What the five sentences leave out, and must. The developmental theory, the metapsychology, and the claim that this process is the origin of thought. Those are Bion's; the five sentences are what a family session needs.

The exercise. Read the passage, write your own five sentences, and then test them against a session: find one moment in your recorded work where a family member put a feeling into you, and describe what you did with it. Most students discover that they evacuated it — into advice, into reassurance, into a change of subject — within ninety seconds. That discovery is the most useful thing this week can produce, because it is the beginning of a skill that no model owns and every model needs.

2.12 Key terms

Term	Definition
Object relations	Internalised working models of self and other formed in early relationships
Projection	Disowning a feeling and locating it in another person
Projective identification	Projection plus pressure, recruiting the other to behave in accordance with it
Splitting	Experiencing people as wholly good or wholly bad, the views held separately
Repetition compulsion	The re-enactment of earlier relational patterns in new relationships
Containment	Holding a feeling without evacuating it into advice, reassurance or action
Transference	The family's feelings towards the therapist, read as data about their relational world
Countertransference	The therapist's feelings towards the family, read as data
Interpretation	A tentative statement linking present behaviour to an unconscious process
Working through	The repeated return to a pattern until it can be held differently
Reflective function	The capacity to understand behaviour in terms of mental states
Family myth	A shared, unquestioned belief that organises a family's account of itself

2.13a The model in three sentences, and what each sentence costs

The matrix row has to be written in the student's own words. Three sentences are enough for a good row, and each carries a cost that the blind-spot column must record.

Sentence 1 — the theory of problem. "Difficulties are maintained by internalised relationship patterns from earlier in the family's history, which are re-enacted in the present, with one member often carrying a feeling the family cannot hold." The cost: the account locates the problem in the family's internal world, which can obscure material conditions, institutions and power.

Sentence 2 — the theory of change. "Change comes through containment and through carefully timed interpretation: the family discovers that a feeling can be held rather than evacuated, and the pattern becomes thinkable rather than compulsive." The cost: the mechanism is slow, and there is little trial evidence for the package as delivered; the clinician must claim component evidence, not outcome evidence.

Sentence 3 — the therapist position. "The therapist is a participant-observer whose own emotional response is data, and whose authority to interpret must be earned in the room." The cost: the position assumes a family able to tolerate and interested in interpretation, and it has historically under-examined its own power.

How to use the three sentences. In the matrix, in the viva, and in the first supervision session of a placement, because they are short enough to be remembered under pressure and complete enough to be challenged. A student who can say them from memory, and who can name the cost of each, has level 1 and is ready for level 2.

2.13 Self-test

1. What are the five core concepts of the school?
2. What is the difference between projection and projective identification?
3. Give the four rules for an interpretation that will land.
4. Why is the therapist's countertransference treated as data?
5. What does "containment" mean, and why is it a mechanism rather than a technique?
6. State the model's theory of problem in one sentence.
7. What are the three separate evidence claims that a good matrix cell distinguishes?
8. Name three cultural assumptions the model carries, and say what each would make hard to see in a three-generational household.

Answers. 1. Object relations; projection; projective identification; splitting; repetition compulsion (with the family as a shared unconscious system). 2. Projection locates a disowned feeling in another; projective identification also pressures the other to behave in accordance with it. 3. Link rather than label; offer rather than pronounce; interpret the pattern rather than the person; go one step beyond what the family knows. 4. Because it is information about the family's relational world, and because it is otherwise a source of enactment. 5. Holding a difficulty without converting it into advice or action; it is a mechanism because the family's experience of having a feeling survived is itself the change. 6. Difficulties are maintained by unconscious processes and internalised relationship patterns that are re-enacted in the family now, with one member often carrying a feeling the system cannot hold. 7. The package; the closely related individual modality; and the components. 8. That the individual internal world is the primary unit;

that the therapist's authority to interpret is welcome; that the healthy family is one of separate individuals with clear boundaries.

2.14 Reflective task

Think of a family you know well — your own, or one from placement — in which one member is described by the others in a way that seems to say more about the describers than the described. Write 300 words on what the description does for the family, and then write the sentence you would say as a therapist, following the four rules. Bring both to the seminar. Most students find that the sentence is much harder to write than the analysis, which is the point of the week.

2.15 Readings for this week

- **Scharff & Scharff**, Object Relations Family Therapy, chapters 1–4, for the model's own account of its method.
- **Scharff & Savege Scharff (eds.)**, The Primer of Object Relations Therapy, for a shorter introduction.
- **Bion (1962)**, Learning from Experience, for containment — difficult, and worth two hours.
- **Winnicott**, any of the collected papers on the holding environment, read for the language rather than the theory.
- **Fonagy & Target**, on reflective functioning and mentalisation, for the component that has the best evidence.
- **Shedler (2010)**, "The efficacy of psychodynamic psychotherapy", American Psychologist, with the critiques that followed it, for how to read an evidence claim about this tradition — and read the critiques first, because the strongest form of the objection is the one the essay will require you to state.

Next: Week 3 — Bowen's multigenerational theory

Week 3 — Bowen's multigenerational theory

The question. How is anxiety transmitted across generations, and who in the system absorbs it?

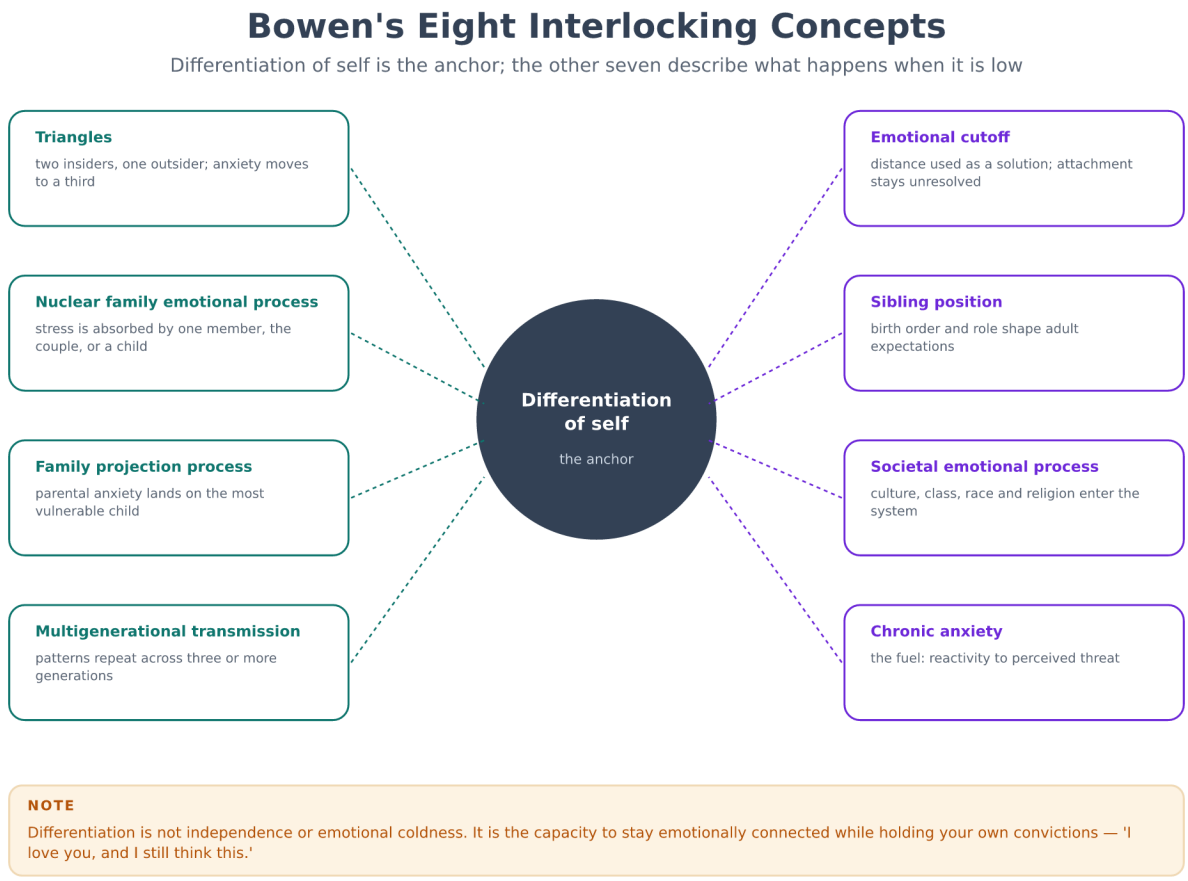


Figure 3.1 — Bowen's eight interlocking concepts

3.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Anxiety as the organising concept	Two families, same event, different anxiety	Predict which will absorb it in a child
10–30	The eight concepts	Build the diagram on the board, one concept at a time	Add the eighth before it is named
30–45	Differentiation, defined precisely	Distinguish from autonomy, from coldness, from individuation	Score three statements on a differentiation scale
45–60	Triangles, demonstrated	A live triangle with three volunteers	Name the triangle in a transcript
60–72	The coach, not the fixer	Demonstrate coaching vs advising	Coach a peer for six minutes
72–85	Detriangling	The three moves, practised	Role-play with feedback
85–90	Critique and matrix row	What the theory cannot see	Fill the row

3.2 Core concepts

Bowen's theory is a system of eight interlocking concepts, and it is taught badly when they are presented as a list. They are interlocking because each one explains a different part of a single process: anxiety, generated in a family, is absorbed by the most vulnerable member, and the pattern that does the absorbing is transmitted down the generations.

Differentiation of self. The capacity to remain a separate thinking person while emotionally connected to others — to be close without fusing and separate without cutting off. It is measured on a scale from low (where feeling and thinking are fused, and the person's functioning is governed by the emotional atmosphere) to high (where the person can hold a position under pressure). Two clarifications that students need.

Differentiation is not autonomy. An autonomous person may be entirely cut off and quite undifferentiated, in Bowen's sense: the person who leaves and never returns has not resolved the fusion, they have moved away from it.

Differentiation is not coldness. The most differentiated person in a family is often the warmest, because they can stay close to a difficult person without being destabilised by them.

Triangles. A two-person system under tension recruits a third. The third may be a person (a child, a mother-in-law, a lover), an activity (work, drink, a sport) or a symptom. The triangle stabilises the dyad by moving the tension onto the third point, and it is the basic unit of Bowen's clinical thinking: **the family's emotional process is always triadic, and the therapist who sees only the dyad will miss where the tension has gone.**

The nuclear family emotional process. The four ways a couple handles anxiety: marital conflict, dysfunction in one spouse, projection onto a child, and emotional distance. Most couples use more than one, in sequence, and the sequence is predictable — conflict, then distance, then a child who becomes the focus.

The family projection process. The mechanism by which parental anxiety is transmitted to a child: the parent perceives the child as vulnerable, treats the child as vulnerable, and the child comes to function as vulnerable. It is not blame; it is a description of a process that runs largely outside awareness, and it is the closest Bowen comes to the object-relations idea of projective identification (Week 2).

Emotional cutoff. The way a person manages unresolved attachment by reducing contact — leaving, emigrating, refusing to speak, moving to another city. Cutoff looks like independence and functions as avoidance; it is transmitted, so that the children of a cut-off person often have no relationship with half their family.

Multigenerational transmission. The process by which differences in differentiation between children are transmitted and amplified across generations, so that a small difference in one generation may become a substantial difference three generations later. This is the concept that gives the theory its time frame and its genogram.

Sibling position. The idea, drawn from Toman's work, that position in the sibling group shapes personality and expectations, and that spouses often marry someone in a

complementary position. It is the theory's weakest empirical component, and the one students most enjoy.

Societal emotional process. Bowen's extension of the theory to whole societies, in which anxiety produces regression in the social contract. It is speculative, it is fascinating, and it is not examinable as a clinical method; mention it in the essay and get on with the family.

Chronic anxiety. Anxiety that is not attached to a specific threat and that circulates in the system, driving the processes above. Acute anxiety is a response to an event; chronic anxiety is a property of the system, and it is what the therapist treats.

3.2a The eight concepts as one process

Bowen's concepts are usually presented as a list, and the list obscures the argument. The argument is a single sentence, and each concept is a part of it.

The sentence. A family is an emotional unit; the unit's chronic anxiety is transmitted to the most vulnerable member through the patterns the family habitually uses, and the differences in how members handle that anxiety are amplified across generations.

Where each concept sits in the sentence.

Chronic anxiety is the fuel. It is the quantity that the system must handle, and it does not need a cause; it circulates.

Differentiation of self is the capacity that determines how much anxiety a person can carry without becoming symptomatic or passing it on. It is the theory's individual-level variable, and the distribution of differentiation across the family is what makes some members more vulnerable.

Triangles are the mechanism of distribution. Anxiety that is too much for a dyad is moved to a third point, and the third point is chosen by availability, not by fault: the child who is at home, the relative who listens, the son who has always been the sensitive one.

The nuclear family emotional process is the repertoire of the couple — the four ways a two-person system handles its anxiety — and it is where the distribution begins.

The family projection process is the specific route by which anxiety reaches a child, and it is the concept that explains why the identified patient is usually a child rather than an adult.

Emotional cutoff is the route by which anxiety is managed by distance, and it is why the theory insists on the genogram: the person who left is still in the system.

Multigenerational transmission is the time dimension: the amplification of small differences over generations, which is why the theory needs three generations to be testable.

Sibling position is a minor, weaker addition, best understood as an attempt to account for the specific shape of personality within a family.

Societal emotional process is the theory extended to a whole society, and it is speculative.

Why the integration matters for practice. A therapist who has learned the concepts as a list will diagnose a family with a checklist ("this family has triangles and a cutoff"). A therapist who has learned the process will see one thing happening — anxiety moving through a system — and will ask where it is now, where it was before, and what would let one member hold more of it without passing it on. The second therapist can work with a family in which none of the textbook features is present, which is most families.

The test of understanding. Explain to a colleague, in ninety seconds and without the vocabulary, what this theory claims happens in a family under stress. If the explanation requires the word "triangle", the understanding is verbal; if it does not, it is real. The same test works for every model in this course, and it is worth applying to each.

3.3 Differentiation, tested

Because differentiation is the theory's central construct and the easiest to romanticise, students should be able to recognise it in behaviour rather than in a definition.

Four statements. Score each for differentiation (high, medium, low), and explain.

> (a) "I love my mother, and I will not discuss my marriage with her." (b) "I have not spoken to my father in nine years and I am at peace with it." (c) "Whatever my husband decides is fine with me; I do not want to cause trouble." (d) "My sister's anger frightens me, so I tell her what she wants to hear and then do what I think is right."

The answers the theory implies. (a) High: contact maintained, position held, no cutoff and no fusion. (b) Low, despite the appearance of independence: cutoff is not differentiation, and "at peace" is often the story a cutoff tells about itself. (c) Low: the self has dissolved into the other's preference. (d) Low-to-medium: the position is held, but secretly, which is a common form of undifferentiation — the person cannot say the position in the room.

Why the exercise matters. Students new to the theory tend to score (b) highest, because it looks like autonomy. The correction — that cutoff is the classic undifferentiated move — is one of the most useful corrections in the course, because it also describes a pattern common in therapists, who sometimes choose a profession in order to manage a family they have left.

The fifth statement, for discussion. "I can tell my father that I disagree with him, hear his anger, and still spend Sunday with him." This is the theory's picture of differentiation, and it is a behaviour rather than a feeling. Note that it is also a sentence very few people can say about a parent, which tells you why the theory's therapy takes as long as it does.

3.3a Four families, one anxiety: predicting who absorbs it

An exercise that teaches the nuclear family emotional process better than any definition. Four families face the same event; predict where the anxiety will land, and why.

Family 1. A couple married twelve years, both working, one child (11). The husband's company announces redundancies; his department is at risk. Neither partner discusses worry well; both are practical. Prediction: the anxiety is most likely to appear as marital

conflict about money, or as distance — longer hours, separate evenings — and only if those routes are closed will it appear in the child, typically as a school or sleep difficulty within a term.

Family 2. A couple married eight years, one child (4) who has always been "delicate". The mother's own mother is seriously ill. Prediction: the child's delicacy is already the family's established third point; the new anxiety will intensify the existing projection, and the presenting complaint in six months will be about the child's health or behaviour rather than about the grandmother's illness.

Family 3. A widowed mother, 58, and her son, 27, who lives at home and has never left. She has just retired. Prediction: the retirement removes the structure that kept the dyad's intensity tolerable; the anxiety will appear as the son's stagnation (under-functioning) and the mother's increasing management of his life (over-functioning), and it will present as a problem about his career or his marriage prospects.

Family 4. A couple with three children (16, 13, 8) whose eldest has just left for university. The parents have not been alone together for eighteen years. Prediction: the departure of the over-functioning eldest removes the buffer; the anxiety will appear as marital distance or conflict, and the second child may become the new focus — which the family will experience as a new problem in the middle child rather than as a shift in the system.

What the four predictions share. In each, the anxiety does not disappear; it moves. The model's clinical value is that it tells the therapist where to look for the movement, and that it predicts a presenting complaint that may have nothing to do with the original stressor. A therapist who only treats the presenting complaint will treat the third point and leave the anxiety where it was.

The follow-up question for each family. "If this had not happened, what would the family have been worried about?" The answer names the chronic anxiety, which is what the therapy is actually for.

3.4 Triangles, and how to see one

A triangle is not a coalition. A coalition is two against one; a triangle is a three-person configuration in which tension between two is displaced onto the third. The distinction matters because they require different interventions.

Four common triangle shapes, with the diagnostic question for each.

The child in the middle. The spouses' tension is displaced onto a child, who is either the problem or the solution. Question: "When the two of you disagree, what happens to the children?" The family will describe a child who intervenes, a child who becomes ill, or a child who is sent to a grandparent.

The outside third. A mother-in-law, a lover, a friend, a therapist, or an activity. Question: "When you and your wife are not getting on, who do you talk to?" The answer identifies the third point.

The distant third. Work, drink, a screen, a sport. The third point is not a person, which is why it is often missed. Question: "What happens in the evening when the two of you have not spoken?" The answer frequently names a room, a telephone or a bottle.

The professional triangle. The therapist recruited as the third point — the family member who telephones between sessions, the one who wants to talk alone, the one who says "you understand me". Question, asked of oneself: "Whose side am I being invited to join, and what would it cost me to refuse?" This is the triangle the trainee is most likely to enter and least likely to notice.

How to name a triangle without entering it. The three moves of 3.6, in one sentence each: describe the configuration neutrally ("when the two of you are not speaking, he is the one who carries it"), ask about the person's own reactivity ("what happens inside you when your mother says that?"), and refuse the invitation to adjudicate ("I am not going to say who is right; I am interested in what happens between the three of you"). The refusal is the intervention.

3.4a The matrix row, written

The row a good student would hand in, with the two extra columns the course requires.

Dimension	Bowen's multigenerational theory
Theory of problem	Chronic anxiety in an emotional unit is transmitted through triangles, the family projection process and cutoff, and is absorbed by the most vulnerable member; differences in differentiation are amplified across generations
Unit of focus	The multigenerational emotional system, with the triangle as its basic unit and the genogram as its map
Theory of change	Lower reactivity and raise differentiation in at least one member, who then changes the system by changing their own functioning in it
Therapist position	Coach: neutral, interested, thinking, not responsible for the family's decisions; often working with one motivated person
Signature techniques	Genogram; detriangling (name, ask about reactivity, refuse to take a side); coaching; experiments in contact
Evidence base	Thin for the therapy — very few trials; stronger for the construct of differentiation, which has correlational support and validated measures
Cultural assumptions	That the client has the freedom and privacy to change their own functioning; that differentiation rather than interdependence is the desirable end
Blind spot	Slow; can under-weight structure, gender, money and institutions; can mistake a coaching relationship for a therapy; may intellectualise

What makes the row defensible. The theory of problem is in Bowen's own terms, not the field's; the techniques are derived from it rather than listed beside it; the evidence cell separates the therapy from the construct; and the blind spot names a consequence ("can mistake a coaching relationship for a therapy") rather than a sentiment.

The sentence for the viva. "I use this model as a lens for the multigenerational pattern and as a coaching method for the family member who presents alone; I do not claim trial

evidence for it as a treatment, and I measure whether it is working in my own caseload." That sentence would earn full marks and is worth memorising.

3.5 Therapy: the coach, not the fixer

Bowen therapy is unusual among the models in this course in that it is often conducted with one person, and in that it aims at the person's differentiation rather than at the family's symptom.

The stance. The therapist is a coach: an interested, neutral, thinking presence who helps the client observe the system and their own part in it, and who does not take responsibility for the family's decisions. The coach does not advise, does not arbitrate, and does not become the family's new organiser.

The method, in four steps. Each is a skill that can be practised.

Step 1 — map the system. The genogram of MFT 701, with the eight concepts in mind: where is the anxiety, who absorbs it, where are the triangles, who is cut off, what is the multigenerational pattern.

Step 2 — identify the client's own reactivity. Not the family's behaviour, the client's response: what happens in the client's body and mind when the difficult person does the difficult thing. The question is always "what happens inside you?", asked with genuine curiosity and repeated as often as necessary.

Step 3 — design a small experiment in contact. The client does one thing differently in the system, and observes the result. The classic experiment is staying in contact while holding a position: "tell your mother what you have decided, without defending it, and stay on the telephone for ten minutes afterwards."

Step 4 — debrief the reactivity, not the outcome. The experiment's success is not whether the mother approved; it is whether the client stayed themselves. This is the hardest idea in the model for both clients and trainees, who want the experiment to work in the family's terms.

Why the model uses one person. Because differentiation is a property of a person, and because a family will often send its most motivated member. The model's wager is that a change in one member's functioning changes the system, and the wager is often correct.

What the coach does not do. Does not take the family's side against the client, or the client's against the family. Does not tell the client what to say to their mother. Does not treat the family's symptom as the target of the work. And does not accept the invitation to become the client's new third point.

3.5a Coaching, demonstrated in five exchanges

An excerpted coaching sequence with a client who is not the identified patient — the husband of a couple, seen alone because the wife declined to attend. The presenting complaint is his wife's anger and the couple's distance.

> **C1 Client:** She says I do not listen. I listen. She starts shouting and I leave the room. **C2 Therapist:** What happens inside you when she starts shouting? **C3 Client:** My chest goes tight. I want to get out. **C4 Therapist:** So you leave — not because you do not care, but because it is unbearable. Does she know that? **C5 Client:** (pause) No. She thinks I do not care. **C6 Therapist:** I am not going to tell you who is right about the listening. I am interested in what would let you stay in the room for two minutes when your chest goes tight — two minutes, not twenty. **C7 Client:** I could count. **C8 Therapist:** You could count. And what would you say, if you stayed? **C9 Client:** That I am not leaving because I do not care. That I am leaving because I cannot take it.

Annotations. C2 and C4 are the model's core move: the client's own reactivity named, and re-described in a way that is both accurate and generous. Note that the therapist does not accept the client's framing ("she starts shouting") and does not dispute it; she simply moves the camera.

C6 is the refusal and the experiment in one sentence. It declines to adjudicate the listening dispute, and it proposes a small, testable change in the client's own functioning. The word "two minutes" is doing clinical work: a task the client can actually do, and one whose success does not depend on his wife.

C9 is the outcome of coaching, and it is a sentence the client could not have said at C1. Whether he says it to his wife is a separate question; the model's wager is that the capacity comes first, and that the marriage changes when one member stops leaving.

What the therapist did not do. Did not schedule a joint session to "sort it out"; did not suggest he explain himself to his wife that evening; and did not interpret his leaving as a repetition of his own family's pattern, although it almost certainly is. The interpretation could come later, when the client's capacity to stay has been established; offered now, it would be a theory about him rather than a resource for him.

3.6 Signature technique: detriangling

Detriangling is the practical core of the model, and it is three moves, not one.

Move 1 — name the triangle neutrally. "When your mother criticises your sister, your brother telephones you, and then you telephone your sister. Is that what happens?" The description uses the family's own sequence, names all three points, and contains no evaluation. Naming is not interpreting; it is holding up a mirror.

Move 2 — ask about the client's own reactivity. "And when your brother telephones you, what happens inside you?" The question moves the attention from the family's behaviour to the client's part, which is the only part the client can change.

Move 3 — refuse to take a side, and stay. "I am not going to tell you whether your sister is right. I am interested in what happens to you when you are in the middle of it, and what it would look like to stay in contact with both of them without carrying the message." The refusal is delivered warmly, and the warmth is what distinguishes neutrality from coldness.

The script from the course material. "When your mother criticises your sister, what happens inside you? What would it look like to stay interested without taking a side and without leaving?" Note the three elements: a question about reactivity, a question about staying, and the pairing of "without taking a side" with "without leaving", which is the whole of detriangling in seven words.

What the client usually does. Offers to fix it, offers to confront somebody, or asks the therapist to say who is right. Each is an invitation to enter the triangle, and each is declined with the same question: "what would it look like if you did neither?"

What progress looks like. Not the family's harmony. The client reports staying on the telephone for twenty minutes without defending, without advising and without leaving; the mother reports that something is different; the sister is unaffected, and that is not the measure. The measure is the client's capacity to be in the system without being run by it, and it is observable in behaviour.

3.6a Six experiments in contact, and how to design a seventh

The model's therapy is delivered through small, real experiments in the family system. Six that recur in practice, with the design logic behind each.

Experiment 1 — the position, held. The client tells a difficult relative what they have decided, without defending it, and stays in contact for a stated period afterwards. Design logic: the task tests whether the client can hold a position in the presence of the other's anxiety, which is the operational definition of differentiation. The debrief question: "what happened inside you at the moment you stopped explaining?"

Experiment 2 — the question instead of the rescue. The client, instead of solving a relative's problem, asks them what they intend to do. Design logic: it interrupts the over-functioning/under-functioning pair at the point where the client usually takes over. The debrief question: "what did they do when you did not take it?"

Experiment 3 — the message not carried. The client declines to pass information between two relatives and tells both that they are doing so. Design logic: it removes the client as the third point of a triangle while keeping the relationship with both, which is detriangling in action. The debrief question: "who was most unsettled, and what did you feel responsible for?"

Experiment 4 — the visit without the agenda. The client visits a parent with no purpose other than the visit. Design logic: it tests whether contact is possible without a task, which is often the difference between a fused relationship and a differentiated one. The debrief question: "what did you do in the silences?"

Experiment 5 — the cut-off approached. The client makes contact with someone they have not spoken to for years, with no expectation of resolution. Design logic: cutoff is managed by distance; the experiment replaces distance with contact and lets the client discover what remains. The debrief question: "what did you notice in yourself in the first five minutes?"

Experiment 6 — the observer's diary. The client records, for two weeks, every occasion on which they felt responsible for another adult's feelings. Design logic: it develops the observing capacity before any behavioural change, and it produces the data for the next experiment. The debrief question: "what did you notice about the pattern of the occasions?"

How to design a seventh. Four constraints, in order. It must be small enough to happen this week. It must concern the client's own functioning, not another person's. It must be observable, so that the client can report what happened rather than how they felt about it. It must be safe — no experiment that risks violence, retaliation, eviction or the loss of a needed relationship, and no experiment with a family member who is violent.

What the therapist does not do. Does not design an experiment that requires another person to change, does not promise that the family will respond well, and does not treat a difficult result as a failure. The model's criterion is the client's capacity, not the family's compliance, and the therapist says so before the experiment begins.

3.7 Annotated transcript: detriangling in the room

A nine-turn sequence from a session with the Adeyemi mother, seen alone in session seven. The immediate issue is the mother's phone call with her own mother, aged 71, who lives nearby.

> **T1 Mother:** My mother called last night about my brother. He has not sent money for the rent and she is upset, and she tells me, and then I cannot sleep.
> **T2 Therapist:** So your mother is upset about your brother, she telephones you, and you are awake at two.
> **T3 Mother:** I am always the one. She cannot say it to him.
> **T4 Therapist:** Let me check the shape of it. Your mother is worried about your brother, she tells you, you carry it — and does she tell your sister as well, or only you?
> **T5 Mother:** Only me. I am the eldest.
> **T6 Therapist:** Only you. And when she says these things, what happens inside you? Not what you say — what happens.
> **T7 Mother:** (pause) I feel I have failed. She looked after four of us alone and never complained, and I cannot even manage one phone call.
> **T8 Therapist:** (gently) That is a strong standard you are holding yourself to. Your mother managed four children after your father died — and she is now seventy-one, phoning you at night about a grown man's rent. What would it look like to be interested in her worry without being responsible for it?
> **T9 Mother:** (long pause) I could ask her what she wants me to do. Instead of doing it.

Annotations. T2 reflects the sequence back, and the repetition of the three points — mother, brother, client — is the naming move of 3.6, performed without any evaluation.

T4 is the diagnostic question that establishes the triangle's exclusivity ("only me"), and T5 answers it with the family's own account of the pattern: the eldest carries it. The therapist does not interpret the eldest-child role, which the client has already named.

T6 is the reactivity question, and the phrase "not what you say — what happens" is doing important work: it prevents the client from giving a report of the phone call and requires an account of her inner process.

T7 is the payload: not anger at the mother, not resentment of the brother, but shame — a sense of failure measured against an impossible standard. This is what the therapist was listening for, and it is why the detriangling sequence was built before the interpretation.

T8 holds two facts together: the mother's real achievement and her present behaviour. Note the respect in the first half and the plain description in the second, offered together. The client's own history is now in the room without being used against her.

T9 is the client's own move, not the therapist's, and it is the sign that the detriangling has worked: she has found a position that is neither compliant nor cut off. The therapist's next move is to test it — "when would you say that?" — and to predict what will happen inside her when she does.

What the therapist avoided. She did not say "your mother is being manipulative"; she did not suggest the client confront her brother; and she did not reassure her that she is a good daughter. Each of these would have entered the triangle at a different point. The neutrality is not indifference: T8 is warm, and the warmth is what allows the challenge.

3.8 Laboratory: three moves, three rounds

The week's practical, ninety minutes, in triads with rotation.

Round 1 — name the triangle (10 minutes each). The client describes a family difficulty. The therapist's only task is to describe the three-point configuration neutrally, in the client's own words, and to check it. No questions about feelings, no interpretations, no advice. The difficulty of this round surprises students: most describe the dyad and forget the third point, and most add an evaluative word without noticing.

Round 2 — ask about reactivity (10 minutes each). Continuing the same material, the therapist asks only questions about the client's own response. The prohibition on advice is absolute. The client is instructed to notice when they begin to answer with a report of somebody else's behaviour, and to say so.

Round 3 — refuse the invitation (10 minutes each). The client is instructed to ask the therapist, at some point, to say who is right. The therapist declines, warmly, and stays with the question about the client's part. The observers score the neutrality of the refusal, not its firmness.

Marking rubric.

Criterion	Weight	Full marks
Naming	25%	All three points named, in the client's words, without evaluation
Reactivity questions	25%	Questions about the client's inner response, sustained for at least five exchanges
Refusal	25%	The invitation declined warmly, without coldness or moralising, and the session continues

Criterion	Weight	Full marks
Staying	15%	The therapist does not move into advice, and does not become the new third point
Self-observation	10%	The therapist can say afterwards when they were pulled, and by what

What students find hardest. The refusal. Trainees are trained to be helpful, and refusing to adjudicate feels unhelpful. The correction is the model's central insight: adjudication is what the family already does, and the therapist's value lies in not doing it. The client's rating of the therapist's neutrality is the check.

3.8a The supervision question that keeps the coach honest

The model imposes a discipline on the therapist that is easy to state and hard to practise: do not do for the client what the client's family does for them.

What that means in practice. The over-functioning client will invite the therapist to organise, to advise, to decide, to hold the plan. If the therapist accepts, the therapy reproduces the family's pattern with a new over-functioning partner, and the client's under-functioning is confirmed. The model's answer is the coaching stance, and its safeguard is supervision.

Three questions to ask in supervision, weekly.

Question 1 — what did I do for this client that they could have done? Not a question about kindness; a question about function. If the answer is "I wrote the letter, I telephoned the mother, I made the plan", the therapy is reproducing the pattern.

Question 2 — which of my client's relatives did I become this week? Every therapist is recruited into a role. Naming the role — the rescuer, the organiser, the one who understands, the absent father — is the fastest route out of it.

Question 3 — what did I feel when the client did not change? The feeling is usually the client's own, or the family's, and it is data. A therapist who feels let down by a client who did not complete an experiment is holding something the client's family also holds.

Why the model needs this more than most. Other models have the family in the room, and the family will correct the therapist's function within a session. The Bowen coach may see one person for months, and the only corrective available is supervision. That is a structural risk of the model, and it is why the theory's own literature insists that the therapist do family-of-origin work: the therapist who knows their own over-functioning is less likely to reproduce it invisibly.

3.9 Six errors in Bowen work

1. **Teaching the theory instead of doing it.** Explaining differentiation to a family who came about a school refusal. Fix: the concepts are the therapist's map, not the family's curriculum.
2. **Neutrality as coldness.** Refusing to take a side in a way the family experiences as abandonment. Fix: warmth plus refusal; the two are not in tension.

3. **Cutoff mistaken for differentiation.** Praising a client's decision to have nothing to do with a parent. Fix: ask what happens inside them when the parent's name is mentioned.
4. **The genogram as the therapy.** Six sessions of history and no experiment. Fix: every genogram session ends with one small experiment in contact.
5. **The missing system.** Coaching an individual for months while never inviting the family, when the family is available and the symptom is acute. Fix: the model permits individual work; it does not require it.
6. **The unexamined coach.** Using the model to manage the therapist's own family of origin without noticing. Fix: the family-of-origin work the theory requires of the client, the theory also requires of the therapist.

3.9a Two genograms, one pattern

The model's most useful instrument is the genogram read for transmission. Two versions of the same family, drawn three years apart.

The first drawing. The Adeyemi family, three generations: the maternal grandfather dead at 47 when the mother was 19; the mother the eldest of five; the paternal grandmother dead in 2022; the father the third of six and the only one who left the region. The presenting problem is Amara's school refusal. A student reading this map would notice the two deaths and the two eldest children, and would hypothesise a pattern of eldest children carrying the family's responsibility.

The second drawing, three years later. Amara is now 19 and at university; the presenting problem is the mother's exhaustion and the father's retirement. The map has gained: a maternal aunt who has been seriously ill, whose care has fallen to the mother; a paternal uncle who has returned to the region after twenty years and expects to be received; and a younger brother, now 14, who has begun to refuse school on Mondays.

What the second drawing shows that the first could not. The pattern has reproduced: a school refusal in the younger child, at the point where the mother's care of the wider family has intensified and the father has been at home, unoccupied, for the first time in twenty years. The over-functioning/under-functioning pair has re-formed in a new configuration. The multigenerational transmission is not a mystical claim; it is a description of a family that solves the same problem the same way, and the genogram makes it visible.

The clinical use of the second drawing. The therapist's task is not to prevent the repetition by insight; it is to change one element of it. The specific question is: "when your mother is caring for your aunt, who notices that your brother is struggling?" If the answer is "nobody until the school telephones", the intervention is a small structural change (a weekly conversation between the parents about the children, before the school calls) rather than an interpretation of the pattern.

What the pair of drawings teaches. A genogram is not a fixed document; it is a moving picture, and the most useful thing a therapist can do with a family over years is to redraw it. The model's distinctive contribution is the claim that you can predict, from the earlier

drawing, where the anxiety will go next — and the pair above is an unusually clear demonstration of that claim working.

3.10 Culture, faith and the multigenerational frame

Bowen's theory is unusually well suited to some cultural contexts and unusually poorly suited to others, and a student should be able to say which and why.

Where it fits well. In families where the multigenerational system is strong and present — three-generation households, families with obligations to a wider lineage, families in which the dead are named and remembered — the model's attention to transmission, triangles and cutoff describes what families already know. Many families in Nigeria will recognise the concept of the family projection process without being taught it; they may have a proverb for it.

Where it strains. Three assumptions.

The assumption of choice. The model's therapy asks a client to change their own functioning in the system, which assumes a degree of freedom that some clients do not have: a young wife in a household where her mother-in-law holds authority, a man whose immigration status depends on his brother, a daughter whose school fees are paid by an uncle. The experiment "stay in contact and hold your position" may be unavailable, and the therapist must ask what the client can actually do.

The assumption of mobility and privacy. Telephone calls, separate households and the possibility of a private conversation are privileges. Coaching a client to have a difficult conversation assumes a place to have it.

The assumption that differentiation is the goal. Families that value interdependence, duty and the subordination of individual preference may experience the model's aim as a recommendation of selfishness. The therapist's translation matters: "managing your own reaction" can be offered as a duty to the family — a way of being more useful, not less connected — and that framing is honest as well as strategic.

The spiritual dimension. Many families' multigenerational frame includes the dead, the ancestors and the obligations that survive a death. The genogram of MFT 701 already includes the dead; the Bowen work adds the question of what is still owed. "What does your family still owe your father?" is a Bowen question in a family's own register, and it often produces material that a question about differentiation would not.

> **DEBATE**
> "Bowen's theory asks clients to become the kind of person that Western individualism approves of." Argue for and against, with reference to a family you know.

3.11 Evidence note: clinical theory, thin trials

The honest matrix cell for this model has three parts, and students who state all three do well in the viva.

What the evidence is. Largely clinical and theoretical. Bowen's own writing is case-based; the theory has generated a substantial literature of practice and a body of process research on differentiation, but very few randomised trials of Bowen family systems therapy as a treatment. The evidence for the construct of differentiation is stronger than for the therapy: measures of differentiation and of chronic anxiety have been developed and used in research, and differentiation has been associated with individual and relationship wellbeing in correlational studies.

What can be claimed. That the theory is a coherent, generative account of multigenerational patterns; that the construct of differentiation has some empirical support as a correlate of wellbeing; that the model's insistence on the therapist's own family-of-origin work is a contribution to training that has been widely adopted, including by models with better trial evidence.

What cannot be claimed. That Bowen family systems therapy has been shown in trials to outperform other treatments for any presentation. A clinician who says so is overstating, and a marker will say so.

How to practise it responsibly. Use it as a lens and a coaching method rather than as a claim of efficacy: the genogram, the triangle analysis and the reactivity questions are widely used and compatible with the better-evidenced models; the coaching stance is a defensible way to work with the family member who presents alone; and the outcome measurement required in MFT 701 Week 11 makes the claim testable in your own caseload. That is the honest position, and it is more useful than either enthusiasm or dismissal.

3.11a Where Bowen and structural disagree, on one family

The two models are often taught as alternatives. They are better understood as answers to different questions, and the comparison sharpens both.

The case. The Okonkwos: mother (38), father (41) working in Lagos and home at weekends, Ebuka (9) with escalating morning defiance, a new baby, and the paternal grandmother dead two years ago after caring for Ebuka daily for five years.

The structural reading. The parental subsystem is functionally single in the week and doubled at the weekend; the mother–son dyad has become the household's executive; the father's weekend interventions are uncoordinated with the mother's and confirm his marginality. The first move: a joint session with the parents to agree one position and one sentence about Monday, then a reorganisation of the morning routine so that the parental subsystem is in charge.

The Bowen reading. The father's mother died when he was sixteen and he left home that year; the mother is the eldest of five who raised her siblings after her own father's death. Both manage anxiety by over-functioning, and the family's chronic anxiety has been absorbed by Ebuka since his grandmother's death, which was also the loss of his primary daytime carer. The first move: coach each parent, beginning with whichever is more motivated, to observe their own reactivity and to stay in contact with Ebuka without

managing him; expect the anxiety to move, possibly to the marriage or the new baby, and predict where.

Where they agree. Both see the father's absence as central, both see the mother as overloaded, both see Ebuka as carrying something that belongs to the adults, and both would predict trouble in the marriage if the child's symptom resolved.

Where they disagree. On the target: the structure of the household, or the parents' own functioning. On the unit: the family in the room, or the parent who is most motivated. On the pace: a reorganisation this week, or a change in reactivity over months. On the criterion of success: a decision held and a routine changed, or a parent who can stay in contact with a difficult feeling without managing it.

What a therapist does with the disagreement. Chooses on the family's circumstances: the school's timetable makes a structural first move almost mandatory, and the family's history makes the Bowen work necessary afterwards. States the choice and the review point, as in MFT 701 Week 13. And keeps the other model's prediction in view, because if the structure changes and the anxiety simply moves, the Bowen reading has been vindicated and the second phase is indicated.

The lesson for the matrix. The two rows are not competing descriptions of the same thing; they are different levels of description of one system, and the competent clinician can hold both without choosing a favourite. That capacity is what Week 13 of MFT 701 called deliberateness, and it is what the Week 12 integration assessment in this course examines.

3.12 Key terms

Term	Definition
Differentiation of self	The capacity to stay connected while thinking and acting for oneself
Fusion	The state in which thinking and feeling are so merged that the person is run by the emotional atmosphere
Triangle	A three-person configuration in which dyadic tension is displaced onto a third point
Nuclear family emotional process	The four patterns a couple uses to manage anxiety: conflict, spouse dysfunction, projection to a child, distance
Family projection process	The transmission of parental anxiety to a child, who comes to function as vulnerable
Emotional cutoff	Reducing contact as a way of managing unresolved attachment
Multigenerational transmission	The amplification of small differences in differentiation across generations
Sibling position	The influence of position in the sibling group on personality and partner choice
Chronic anxiety	Anxiety not attached to a specific threat, circulating in the system
Societal emotional process	Bowen's extension of the theory to whole societies under anxiety

Term	Definition
Coaching	The therapist's stance: neutral, interested, thinking, not responsible for the family's decisions
Detriangling	The three moves by which the client is helped out of a triangle without being cut off
Over-functioning / under-functioning	The reciprocal pattern in which one person takes responsibility and another relinquishes it

3.13 Self-test

1. Name the eight concepts.
2. Why is a cutoff not evidence of differentiation?
3. What is the difference between a triangle and a coalition?
4. Give the three moves of detriangling.
5. What are the four nuclear family emotional processes?
6. Why does the model often work with one person?
7. What are the three parts of an honest evidence claim for this model?
8. Name two assumptions of the model that strain in a household where a grandmother holds authority, and say what you would do about each in practice.

Answers. 1. Differentiation; triangles; nuclear family emotional process; family projection process; emotional cutoff; multigenerational transmission; sibling position; societal emotional process (with chronic anxiety as the driver). 2. Because cutoff manages unresolved attachment by reducing contact; the fused attachment remains, and the person is still governed by it. 3. A coalition is two against one; a triangle is a three-person configuration in which dyadic tension is displaced onto a third point, who may not be "against" anyone. 4. Name the triangle neutrally; ask about the client's own reactivity; refuse to take a side and stay. 5. Marital conflict; dysfunction in one spouse; projection onto a child; emotional distance. 6. Because differentiation is a property of a person and families often send their most motivated member; the wager is that a change in one member changes the system. 7. That the theory is coherent and generative; that the construct of differentiation has correlational support; that there are very few trials of the therapy itself. 8. That the client has the freedom to hold a position in the system, and that differentiation rather than interdependence is the goal.

3.14 Reflective task

Draw the triangle you occupy most often in your own family, name all three points, and describe your own reactivity in one paragraph. Then write the sentence you would say to yourself as your own coach, following the three moves of detriangling. Most students find the third move — refusing to take a side — impossible to apply to themselves, because they are already on one. That difficulty is the model's point: the therapist's own triangles are the ones that will run the therapy.

3.15 Readings for this week

- **Bowen (1978)**, Family Therapy in Clinical Practice, especially the chapters on triangles and on the multigenerational transmission process.
- **Kerr & Bowen (1988)**, Family Evaluation, chapters 4–9 — the clearest systematic statement of the theory.
- **Papero (1990)**, Bowen Family Systems Theory, for a short, accurate introduction.
- **Titelman (ed.)**, Differentiation of Self, for the research literature on the central construct.
- **Any published account of Bowen's own coaching of a single client**, read for the stance rather than the content: notice how rarely he advises, how often he asks what the client noticed in themselves, and how long he is willing to wait for the client's own move.

Next: Week 4 — Structural family therapy

Week 4 — Structural family therapy

The question. What is the organisation of this family, and how does its structure maintain the problem?

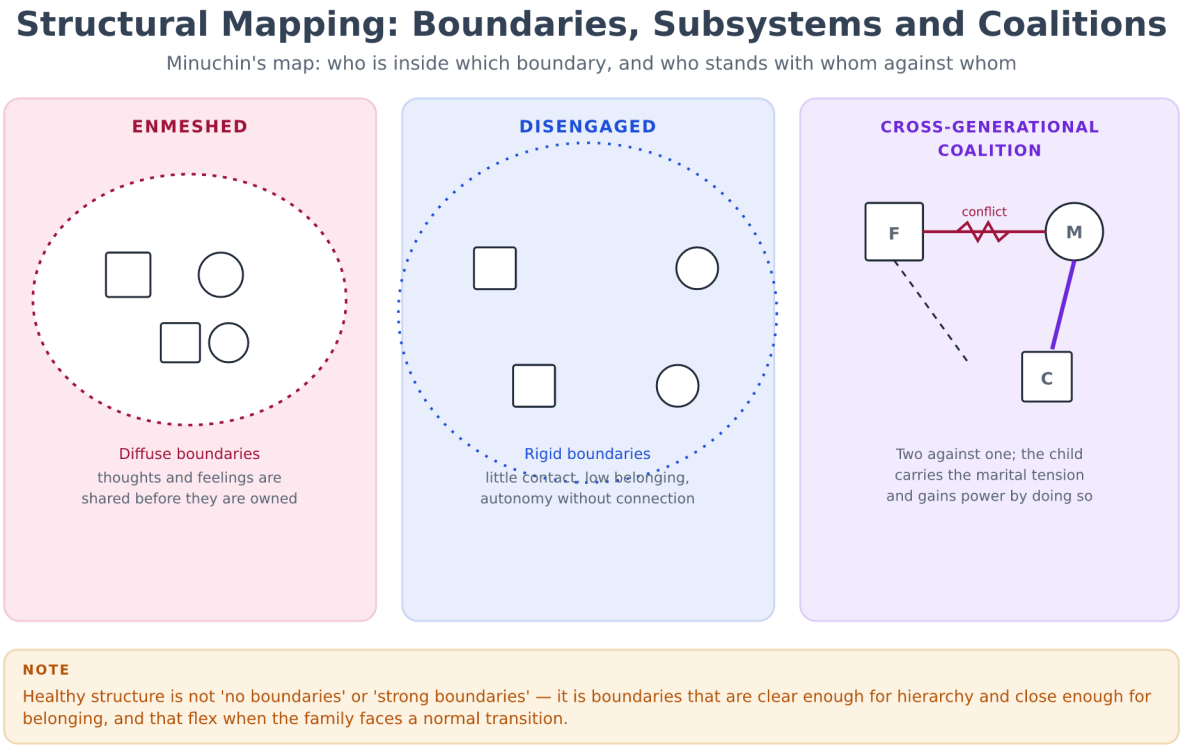


Figure 4.1 — Structural mapping: boundaries, subsystems, coalitions

4.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The idea in one sentence	Structure is what repeats, and it is visible	Name the repeated sequence in a case
10–25	Structure, subsystem, boundary, hierarchy	Draw a family map with all four	Draw their own family's map
25–40	Enactment, demonstrated	Bring a disagreement into the room	Practise one enactment in triads
40–55	Unbalancing and intensity	Two interventions, with the ethics of each	Decide when unbalancing is legitimate
55–70	Joining before challenging	Why the challenge fails without the join	Script the first two minutes of a challenge
70–85	The structural first session	Minute-by-minute architecture	Sequence the moves for the Adeyemis
85–90	Critique and matrix row	What the model cannot see	Fill the row

4.2 Core concepts

Structural family therapy, associated with Salvador Minuchin and the Philadelphia Child Guidance Clinic, is the most teachable of the classical models and the most easily misused. Its concepts are simple; its practice requires timing.

Structure. The repetitive patterns of interaction that organise a family: who speaks to whom, who decides, who is consulted, who is protected, who is left out. Structure is not a diagram imposed on a family; it is what an observer would see if they watched the household for a fortnight. Two consequences follow.

Structure is visible in the room. Every family enacts its structure in the first ten minutes of a session, which is why the model works in the present tense and in the session itself.

Structure is maintained by everyone. The mother who manages alone, the father who is peripheral, the child who intervenes and the grandmother who overrides are all participating in a pattern that none of them designed, and each would describe their own part as reasonable.

Subsystems. Units within the family organised by boundaries: the couple, the parental subsystem, the sibling group, and each individual. The crucial distinction is between the couple subsystem and the parental subsystem — a family can have a strong parental subsystem and a weak couple subsystem, and the two require different work.

Boundaries. The rules governing participation and closeness: who is included, who is excluded, what may be said in front of whom. Boundaries are described as clear, diffuse or rigid. Clear boundaries allow both contact and separateness; diffuse boundaries allow too much of one and too little of the other; rigid boundaries are the opposite. The words "enmeshed" and "disengaged" describe the two extremes, and they are descriptions of a relationship, not of a person.

Hierarchy. The distribution of authority and responsibility, especially between generations. Structural therapy assumes that a family in which children hold executive authority, or in which a grandparent and a parent compete for it, will produce symptoms. The assumption is normative, and the norm is culturally specified — a point developed in 4.10.

Alignment and coalition. An alignment is a joining of members for a purpose ("the two of you agree about the school"); a coalition is a joining against a third ("the two of you against your father"). Coalitions across generational lines are the model's classic pathology, and the child in a cross-generational coalition is the classic identified patient.

Complementarity. The idea that behaviours are reciprocal: the more one member over-functions, the more another under-functions. The concept is useful because it converts a complaint about a person ("he is useless") into a description of a pair ("the two of you have arranged it so that one carries the responsibility and one carries the blame").

The identified patient. The member who carries the symptom, understood not as the site of the pathology but as the place where a structural problem becomes visible. The model's most important claim at the level of practice: **treat the structure, and the symptom will follow** — or, more accurately, treat the structure and you will find out whether the symptom was serving the structure.

4.2a Reading a family's structure in ten minutes

The model's core skill is observation, and observation can be taught as a list of what to watch. Six things, in the order they become visible.

Seating. Who chose where to sit, who sat beside whom, who left a gap, who took the chair nearest the door. Families arrange themselves without deciding to, and the arrangement is data. Ask, later, what would happen if they swapped places — the answer is often an explicit statement of the structure.

Turn-taking. Who speaks first, who speaks most, who is interrupted, who is never interrupted, who translates for whom. The therapist's own interruptions are part of the data: a therapist who interrupts the mother to hear the father is already restructuring.

Speaking for. Who answers a question addressed to someone else. A mother who answers for her son is not necessarily intrusive; she may be the household's designated spokesperson, and the pattern tells you who holds the responsibility for the family's presentation to the outside world.

The referral. Who telephoned, and who decided that the family would come. The person who made the referral often holds the executive position, whatever the family says about its hierarchy.

The silence. Who does not speak, and what happens when the therapist addresses them. A silence that is comfortable is a boundary; a silence that produces tension in others is a coalition; a silence that produces a change of subject is a secret.

The identified patient's position. Where is the symptomatic member in relation to the others, and what happens in the room when they speak? A child who is discussed

throughout the session without being addressed is in a specific structural position, and the position is the diagnosis.

The exercise. Watch ten minutes of any recorded family session, and write the six observations before writing any interpretation. Students who do this find that the structure is legible within five minutes, and that their interpretations change when the observation comes first.

4.3 Therapy: join, then challenge

The model's method has two phases, and the order is not negotiable.

Joining is the therapist's deliberate accommodation to the family's style, rules and language, in order to be accepted as a participant in the system. It is not warmth, though warmth helps; it is a technical operation. The therapist joins by:

adopting the family's language ("you call it stubbornness; I will use that word for now"); respecting its hierarchy (addressing the person who holds authority first, and asking permission where it is expected); accepting its account without endorsing its conclusions ("that sounds exhausting, and I can see why you are angry"); and tracking its style (pace, volume, directness, humour).

Joining without a plan is friendliness. Joining is done so that the challenge can be survived.

Challenging is the therapist's deliberate disruption of the structure: moving the conversation, blocking a coalition, marking a hierarchy, raising intensity until the pattern becomes undeniable, and requiring the family to do something different in the room.

The rule that students must internalise. A challenge delivered without a join is experienced as an accusation, and the family will either comply and hide, or leave. A join without a challenge is a pleasant conversation with no therapy in it. The skill is the sequence and the timing, and it is learned by watching recordings, not by reading.

Three moves of a challenge, in order.

Mark the structure. "I notice that when your wife begins to describe the morning, you look at your phone." The observation is neutral, specific and behavioural.

Change the spatial or conversational arrangement. "I would like you to tell her that, not me." The move is enacted, not discussed.

Hold the intensity. The family will attempt to return to the familiar pattern within seconds. The therapist's task is to keep the new arrangement in place long enough for something to happen — usually no more than a minute or two, and often uncomfortable.

4.3a Joining, technique by technique

Joining is the least glamorous and most decisive skill in the model. Five operations, each observable.

Operation 1 — adopt the family's vocabulary. Use their words for the problem for at least the first session, even when their words are inaccurate. "Stubborn" is the family's

word; the therapist who substitutes "oppositional" in the first ten minutes has told the family that their account is not the one that counts. The therapist's own vocabulary can be introduced later, once the join exists.

Operation 2 — respect the hierarchy you find. Speak first to the person the family treats as the head, ask permission where permission would be expected, and do not assume that the most talkative member holds the authority. In many families the person who telephones is not the person who decides, and a therapist who addresses only the telephoner has joined the wrong subsystem.

Operation 3 — accept the account without endorsing the conclusion. "That sounds exhausting" accepts the experience and does not agree that the child is bad, the husband is useless, or the diagnosis is correct. The distinction is subtle, it is the whole of neutrality in this model, and families test it: a therapist who agrees too quickly is recruited, and a therapist who disagrees too quickly is expelled.

Operation 4 — track the family's style. Pace, volume, directness, humour, formality and the use of silence are all style, and a therapist whose style is markedly different will be experienced as foreign whatever the content. A therapist who speaks quickly with a slow family, or warmly with a formal one, is not joined however accurate the formulation.

Operation 5 — be useful early, in the family's terms. A practical question, a piece of information, an offer to write to the school: something that shows the therapist can be helpful, before the therapist asks the family to change. The model's practitioners are often directive early for exactly this reason.

What joining is not. It is not agreement, not flattery, not adopting the family's view of the identified patient, and not pretending to be a member. It is the construction of a working relationship in which the therapist will be allowed to challenge, and the test of a successful join is that the first challenge is survived.

The diagnostic question. After twenty minutes, ask yourself: if I said something difficult now, would this family stay? If the answer is no, the join is not complete, whatever the warmth in the room.

4.4 Signature techniques

Four techniques carry most of the model's practice. Each is a move in the session, and each has a hazard.

Enactment. The therapist asks two members to deal with each other directly while the therapist watches and coaches from the side. The script: "Say that to her, not to me. I will listen." Why it works: it converts a report about the relationship into the relationship, in the room, where it can be changed. The hazard: an enactment begun before the therapist has joined will produce a fight, and the therapist will be blamed for it. The judgement is about timing, and the marker of good timing is that the family can survive the enactment without the therapist rescuing it.

Unbalancing. The therapist deliberately supports one member — usually the one with less power, or the one who is being scapegoated — in order to shift the system's

equilibrium. The script: "I want to hear from her for a few minutes, and I would like the rest of you to listen without correcting her." Why it works: a system that is stuck in one distribution needs a temporary imbalance to move. The hazard: unbalancing looks like taking sides, and a therapist who unbalances without explaining it, or who unbalances and stays unbalanced, loses the other members. The ethical requirement is that the unbalancing serves the family's flexibility rather than the therapist's sympathy.

Boundary-making. The therapist marks and enforces a boundary in the room. The script: "This is a conversation for the two of you; the children will wait outside for ten minutes, and then I will bring them back and tell them what we decided." Why it works: it enacts the structure that is being proposed, rather than describing it. The hazard: boundary-making that excludes a member without explanation is experienced as exclusion; the explanation is part of the technique.

Intensity. The therapist raises the emotional temperature until the pattern cannot be avoided. The script from the course material: "This is the fourth time you have laughed when your wife described the miscarriage. What is that laugh?" Why it works: some patterns are maintained by the family's ability to keep the conversation at a safe distance, and intensity removes the distance. The hazard: intensity is the technique most likely to harm. It requires a strong join, a clear clinical purpose, and the therapist's confidence that the family can survive it. Used to express the therapist's frustration, it is abuse with a theoretical justification.

4.5 The structural first session

A minute-by-minute architecture, which students can rehearse and which transfers to almost any model.

Minutes 1–5: join and set the frame. Greet each person by name, establish the practical frame (time, confidentiality — see MFT 701 Week 12), and ask the opening question. The opening question is diagnostic: "What brings you here?" is asked to the whole family, not to the person who telephoned.

Minutes 5–15: watch the structure. Do not intervene. Ask each member what they think the problem is, and observe who interrupts, who defers, who speaks for whom, who is silent, and where each person is sitting. The structure shows itself in the first ten minutes to a therapist who is not busy asking a list of questions.

Minutes 15–25: map and mark. Draw the family map — in your notes, and if useful on paper for the family — with subsystems, boundaries, and alignments. Then mark one structural feature aloud, neutrally: "I notice that when your mother begins to describe the morning, you look at your phone."

Minutes 25–40: the first enactment. Move two members into direct conversation about something concrete, and coach from the side. This is where the model's work happens, and it is where the session is most likely to go wrong.

Minutes 40–50: one structural change. Agree something that changes the arrangement: a decision the parents make together, a boundary about who speaks to the

school, a rule that holds for a week. It must be specific, behavioural and owned by the family.

Minutes 50–60: close, with the change named. Summarise the structure you saw, the change you are proposing, and the task. Ask each member whether they can do it, and record what they said.

What the architecture is for. It front-loads observation and back-loads intervention, which is the opposite of what anxious trainees do. The structure cannot be challenged before it has been seen, and it cannot be seen while the therapist is talking.

4.5a The structural map, drawn and used

The map is a working instrument, not an illustration. Four symbols, one page, and three questions.

The symbols. A rectangle for each person, with the identified patient marked. A line between members for a relationship, with a double line for a close one, a broken line for a distant one, and a zigzag for a conflicted one. A heavier line around a subsystem that holds authority. An arrow for a coalition.

The three questions the map answers. Where is the authority? (the heavier line). Where does the tension go? (the zigzag and the arrow). Who is outside? (the person with broken lines and no subsystem).

A worked map, for the Okonkwos. The mother and Ebuka are enclosed in a subsystem marked with the heavier line, which shows that the executive authority in the week is the mother–son pair rather than the parental pair. The father is connected to the mother by a broken line labelled "weekends", and to Ebuka by a dotted line labelled "Friday evening". The new baby is attached to the mother with a double line and no connection to the father, which is a prediction about the next presenting problem. The maternal aunt is drawn outside the household with a double line to the mother, labelled "support", and no connection to anyone else.

What the map predicts. That the father's attempts to intervene will be experienced as an intrusion, because he is structurally outside the executive subsystem. That the baby will become the next focus of the mother–child pair. And that the aunt is the family's only external resource, which makes her both an asset and a risk if the relationship fails.

How to use the map with the family. Draw it on paper in front of them, roughly, and ask them to correct it. The corrections are the assessment, and the moment when a member says "no, put him closer" is a moment of structural information that no questionnaire would produce.

What the map is not. It is not a diagnosis and not a plan. It is a description of the arrangement that the therapist believes is maintaining the problem, drawn so that it can be checked, challenged and changed.

4.6 The Adeyemi first move, worked in full

The course material gives the structural reading and two moves. Here is the sequence, with the reasoning and the likely obstacles.

The reading. The parental subsystem is weak — functionally single in the week, doubled and uncoordinated at the weekend. The mother–daughter coalition crosses the generational line: Amara negotiates with her mother, and her mother's authority is contested by the father's weekend interventions. The father is structurally peripheral and reacts with criticism when pulled in, which confirms his marginality. The younger brother is peripheral, observant, and describes the household's emotional temperature accurately.

Move 1 — the parents alone. A session with the two parents, by telephone if the father cannot travel, to plan one consistent rule and one shared evening. The purpose is structural: to create a parental subsystem that exists before the session with Amara, so that there is something for her to encounter.

The obstacle. The father may experience this as being summoned to be told off. The structural answer: join first — acknowledge the cost of working away, and ask him what he would want his daughter to know about his work. Then the task: "I want the two of you to decide one thing together, and I want the two of you to tell her, in the same words."

Move 2 — the joint session. The parents tell Amara the plan together, while the therapist blocks her attempts to negotiate with her mother alone.

The script. "Amara, your parents have decided something and they are going to tell you together. I want you to listen first and then say what you think." Then, when Amara turns to her mother: "Turn back to both of them. This is their decision."

The obstacle. Amara will test the coalition — she will appeal to her mother, and her mother will respond. The therapist's task is to block the appeal without humiliating either of them: "I know it is easier to ask your mum. Ask them both."

Move 3 — the follow-up. A task that requires the parents to act together during the week, and a review at which the therapist asks the parents first, and asks the brother what he noticed. The brother's answer is the model's outcome measure: if the structure has changed, the household's emotional temperature changes, and the peripheral member is usually the first to report it.

What would falsify the reading. If the parents agree a rule and hold it for two weeks and the mornings do not change at all, the coalition is not the maintaining mechanism (compare MFT 701 Week 13's testability requirement). The next hypothesis would be the mother's grief and the daughter's role as her companion — a Bowen or object-relations reading — and the plan would change accordingly.

4.6a Two structural first sessions, one that works and one that does not

The session that works. A family of four: parents, a 15-year-old son referred for "aggression", and a 12-year-old daughter. The therapist joins for twenty minutes, noting that the mother answers every question, the father looks at the mother before speaking, and the son has not been addressed. In minute nineteen the therapist says: "I would like to hear from your son for a few minutes, and I would like the two of you to listen without correcting him." The parents agree. The son speaks for four minutes and describes a household in which his father is "always at work and then always angry". The therapist says, to the father: "He has just told you something. What do you want to say to him?" The father, after a long silence, says: "I did not know he thought that." The session ends with a task: father and son, one hour, on Saturday, without the mother. The mother objects; the therapist acknowledges the objection and holds the task.

Why it works. The join was long enough. The first move was a boundary: it moved the conversation from the mother–therapist dyad to the father–son dyad, which is where the structural problem was. The task enacted the boundary in the week. And the therapist held the arrangement through the mother's objection, which is the unbalancing that the system needed, delivered without criticising her.

The session that does not work. The same family, a different therapist. Ten minutes of joining. At minute eleven the therapist says: "I think the problem is that the two of you are not united as parents, and your son is running the house." The mother defends herself, the father agrees with the therapist, the son becomes silent, and the therapist spends the remaining forty minutes managing the conflict she has produced. The session ends with an apology from the mother and no task.

Why it fails. The challenge came before the join could survive it. The formulation was delivered as a verdict ("I think the problem is") rather than enacted. The father's agreement positioned the therapist in a coalition against the mother, which confirmed the structure rather than changing it. And the identified patient was discussed rather than addressed, which is the pattern the family came with.

The difference, in one sentence. The first therapist changed the arrangement and let the family discover the problem; the second described the problem and let the family defend itself. That sentence is the model, and it is the reason the laboratory is structured around three attempts rather than around a discussion of technique.

4.7 Annotated transcript: an enactment that works

A twelve-turn sequence from a first session with the Ogundipes, whose presenting complaint is the wife's "anger" and the husband's "distance". The therapist has joined for twenty minutes and is now making the first structural move.

> **T1 Therapist:** I want to try something different. Mrs Ogundipe, you have told me three times what happens when you ask him about the money. I would like you to tell him, not me, and I will sit back.
> **T2 Wife:** (to therapist) He will not answer.
> **T3 Therapist:** Maybe. Tell him anyway. (waits)
> **T4 Wife:** (to husband, after a pause) When I ask you about the money you go quiet, and then I am shouting at nothing.
> **T5 Husband:** I am not going quiet. I am thinking.
> **T6 Wife:** You are thinking for three days?
> **T7 Therapist:** (to husband) Answer her. Not me.
> **T8 Husband:** (long pause) I do not know what to say, so I say nothing.
> **T9 Wife:** And I am supposed to guess that?

> **T10 Therapist:** (to wife) Ask him what he needs from you in that moment.
> **T11 Wife:** (pause) What do you need from me when you go quiet?
> **T12 Husband:** (quietly) Ten minutes. Not a solution. Ten minutes.

Annotations. T1 announces the move, names the pattern the therapist has observed ("three times"), and instructs the two members to address each other. The therapist's withdrawal — "I will sit back" — is the structural operation: the therapist leaves the executive position so that the couple can occupy it.

T2 is the first test. The wife addresses the therapist, which is the pattern the enactment is designed to interrupt. T3 does not argue, does not reassure, and does not repeat the instruction at length; it holds the arrangement with two words and a wait. Holding is the technique, and the wait is the intervention.

T6 is the moment the escalation begins, and T7 redirects the husband without criticising the wife — a small act of unbalancing toward the more passive member, in the service of the enactment rather than of either person.

T11 is the payoff, and it is the therapist's prompt that produces it: a question that requires the wife to be curious rather than accusatory, addressed to her husband rather than to the therapist. T12 is the sentence the couple came for, and neither could have said it in a session conducted as a series of reports to the therapist.

What the therapist did not do. Did not interpret the husband's silence (an object-relations move), did not teach a communication skill (a behavioural move), and did not ask about the couple's families of origin (a Bowen move). Each of those is a legitimate intervention in its own model; each would have interrupted the enactment.

What the therapist watched for. Whether the couple could survive sixty seconds of direct conversation without the therapist rescuing them. They could, just, and the enactment was extended. If they had not been able to, the therapist's task would have been to shorten the next enactment, not to abandon the technique.

4.7a When the enactment fails, in three ways

Enactments fail more often than they succeed in training, and the failures are instructive.

Failure 1 — the couple addresses the therapist. Both members continue to speak to the therapist while ostensibly speaking to each other, and the therapist becomes the relay. The correction: do not paraphrase and do not answer. Say "tell her" and wait, however long the silence. The wait is the intervention, and trainees find it almost unbearable: the average trainee breaks a silence at four seconds, and the family needs twelve.

Failure 2 — the enactment escalates into a fight. The volume rises, one member threatens, and the therapist intervenes to protect. The correction, in the moment: stop the enactment, name what happened without blame ("this is getting hot; let us pause"), and return to it later in shorter form. The correction, in the design: shorter enactments, a narrower topic, and an explicit agreement before beginning — "say one sentence, then let him answer, and I will keep the time."

Failure 3 — the enactment is polite and nothing happens. The couple performs a calm exchange, agrees, and the session ends pleasantly. The diagnosis: the topic was too safe, or the therapist's join was so comfortable that nothing was at stake. The correction: choose a subject on which the couple genuinely disagree, and state the stakes — "you have told me two different things about Saturday; I want to hear you disagree about it."

The principle behind all three corrections. An enactment is a controlled exposure to the relationship itself. It needs enough reality to produce the pattern, and enough structure to survive it. The therapist's judgement is entirely about that balance, which is why the technique cannot be learned from a book and why the laboratory uses three attempts rather than one.

4.8 Laboratory: one enactment, three attempts

The practical runs in triads with a strict structure, because enactment is the technique trainees most often perform badly and most often mistake for a conversation.

Setup. A role-played couple with a live disagreement (money, a mother-in-law, a child's school). The therapist has five minutes to join, and then must attempt one enactment.

Attempt 1 — no join. The therapist moves to the enactment within ninety seconds. The couple deflects, the therapist insists, and the exercise is stopped at four minutes. The observers note what happened in the couple's bodies and voices.

Attempt 2 — join, then enactment, then rescue. The therapist joins for five minutes, begins the enactment, and then intervenes after twenty seconds to smooth the conflict. The exercise is stopped. The debrief question: "what did the rescue do to the arrangement?"

Attempt 3 — join, enact, hold. The therapist joins, moves to the enactment, and holds the arrangement for two minutes without rescuing, coaching only when a member addresses the therapist instead of the partner.

Marking rubric.

Criterion	Weight	Full marks
Joining	20%	The family's language, hierarchy and style accommodated before any challenge
The move itself	25%	A clear, announced instruction to address each other, with the therapist visibly withdrawing
Holding	25%	The arrangement maintained for at least ninety seconds; redirects rather than rescues
Coaching from the side	15%	Prompts that require curiosity or specificity, not prompts that supply the content
Debrief	15%	Can describe what the enactment revealed and what would have made it unsafe

The commonest failure, and why it matters. Rescuing. Trainees intervene at the first sign of discomfort, and the intervention converts the enactment back into a report. The correction is not to become comfortable with distress; it is to distinguish distress that the family can survive from distress that requires stopping, and to hold the first while stopping the second. That distinction is the whole of the technique, and it is learned by watching recordings of oneself.

4.8a The matrix row, written

Dimension	Structural family therapy
Theory of problem	Difficulties are maintained by the family's organisation: unclear or inverted hierarchy, diffuse or rigid boundaries, and cross-generational coalitions that recruit a member into adult conflict
Unit of focus	The whole family as a structure with subsystems and boundaries, observed in the session
Theory of change	Restructure in the room; action precedes insight, and a different arrangement produces a different experience
Therapist position	Active director and stage-manager who joins and then challenges; uses self as a lever, explicitly
Signature techniques	Enactment, unbalancing, boundary-making, intensity, spatial arrangement
Evidence base	Moderate to strong for child and adolescent behaviour problems, adolescent substance misuse and eating disorders, usually as a component of a family-based programme; thin as a stand-alone treatment for adult and couple presentations
Cultural assumptions	That a two-parent hierarchy with clear generational boundaries is the healthy form; that the household is the relevant unit; that the therapist has the mandate to restructure
Blind spot	Can impose a norm the family does not hold; can be experienced as controlling; under-attends to inner life, history, gender and material conditions

The viva sentence. "I use structural work when the maintenance pattern is organisational and can be acted on in the room, with the caveat that the hierarchy I strengthen must be the family's own and must not be maintained by fear — and I measure whether the restructuring holds in the week."

4.9 Six errors in structural work

1. **The challenge without the join.** Moving to restructuring in the first ten minutes. Fix: twenty minutes of observation and accommodation before the first challenge, and no exceptions for experienced therapists.
2. **The rescue.** Intervening at the first sign of conflict, converting the enactment back into a report. Fix: hold for ninety seconds; redirect only when a member addresses the therapist instead of the partner.
3. **The imposed norm.** Restructuring a family toward a two-parent, hierarchical, Western nuclear arrangement that the family does not hold and cannot enact. Fix: ask what the family's own hierarchy is, and work with it.
4. **Unbalancing that becomes siding.** Supporting one member for so long that the others experience the therapist as their opponent. Fix: state the purpose of the unbalancing, keep it temporary, and rebalance before the session ends.
5. **The missing symptom check.** Restructuring for six sessions and never asking whether the presenting problem has changed. Fix: the review question at every third session, with the family's own measure.
6. **Intensity as irritation.** Raising the emotional temperature because the therapist is frustrated. Fix: intensity requires a stated clinical purpose and a strong join; without both, it is abuse with a rationale.

4.10 Culture, faith and the structural norm

This model carries the clearest normative picture of a family in the whole course, and the picture is culturally specific. A student who cannot critique it should not use it.

The norm. Minuchin's model assumes a family with a parental subsystem that holds executive authority, clear boundaries between the generations, and children who are not involved in adult conflicts. The picture is of a nuclear household with two parents, in a society where the state, the school and the clinic are external.

Where the norm does not fit. Four common configurations.

The three-generational household. When a grandmother holds executive authority over the children and the mother implements it, the structural therapist's "strengthen the parental subsystem" may mean removing the grandmother's authority — which is a cultural intervention disguised as a structural one. The correct move is to ask what the family's hierarchy is, and to work with its own distribution of authority: "In this house, who decides about school? And who does your mother consult?" The structural problem is not that the grandmother has authority; it is that the authority is unclear, or contested, or exercised without coordination.

The household divided by migration. A father working in another city is structurally peripheral by circumstance, not by disengagement. The model's language ("peripheral father") can imply a choice. The correct formulation names the arrangement and its cost, and works with the arrangement rather than against it — for example, by building a

parental subsystem that operates by telephone at an agreed time, and by making the weekend a structural event rather than an afterthought.

The extended family as the executive. In many families the decisions about a child are made by a lineage group — the father's brothers, the mother's eldest sister, a family head. A structural therapist who works only with the household will restructure a subsystem that does not hold the authority. The correct move is to map the decision-making network and to invite its relevant members, or to work with the household's account of them.

Gender and authority. The model's "hierarchy" can be read as an endorsement of the father's authority, which is harmful where the father's authority is coercive. The ethical floor of MFT 701 Week 12 applies: the model must not be used to strengthen a hierarchy that is maintained by fear.

How to use the model well across cultures. Three practices.

Ask for the family's own map. "If I drew this family the way you see it, who would be at the top, who would be at the sides, and who would be in the middle?" Families produce a structure, and the structure is the one to work with.

Distinguish clarity from content. The model's real claim is not that a particular hierarchy is correct but that unclear hierarchy produces symptoms. That claim survives translation: a family in which nobody knows who decides, or in which two people both believe they decide, will produce difficulty, whatever the cultural form.

Let the family judge the change. A restructuring that the family experiences as a loss of its own values will not hold. The test is not whether the therapist's map is elegant but whether the family can enact the new arrangement without feeling that it has betrayed itself.

> **DEBATE**
> "Structural family therapy is the most culturally risky model in this course." Argue for and against, using the three-generational household as your case.

4.10a Two critiques, stated in their strongest form

Critique 1 — the model is manipulative. The therapist forms a diagnosis that is not shared with the family, deliberately unbalances the system, raises intensity without consent, and arranges the family's seating and conversation to produce a predetermined change. In its strongest form: the model treats the family as an object to be reorganised, and its techniques are effective precisely because the family does not know what is being done.

The strongest answer. The model's practitioners argue that the family is told what is happening in the room ("I want you to say that to her, not to me"), that the restructuring is offered rather than imposed (families refuse tasks, and the refusal is information), and that the alternative — a therapist who explains a formulation and asks the family to agree with it — presumes a verbal, middle-class competence that many families do not have. The response is respectable and incomplete: the charge of manipulation has force wherever

the therapist's purpose is concealed, and the corrective is transparency about the move, not concealment of it.

Critique 2 — the model imposes a cultural norm. The theory assumes a hierarchy in which parents hold executive authority, a household with two parents present, and clear generational boundaries. In its strongest form: the model treats the Western nuclear family as healthy and everything else as an approximation of it, and its interventions therefore pathologise extended, transnational and matrifocal families.

The strongest answer. The theory's actual claim is narrower than its reputation: it claims that unclear hierarchy produces symptoms, not that a particular hierarchy is correct. That claim survives translation, and the model can be used with a three-generational household by working with the hierarchy the family actually holds. The response is again respectable and again incomplete: the model's case examples and its normative vocabulary carry the stronger claim, and a clinician who uses the model without auditing that vocabulary will reproduce it.

What the two critiques have in common. Both concern power: the therapist's power to define and reorganise, and the model's power to define what a family should look like. The mature use of the model requires the therapist to be explicit about both, which is the same requirement MFT 701 Week 12 set for every model in the field.

4.11 Evidence note: the strongest evidence among the classical models

Structural family therapy has better trial evidence than most of the classical models, and the evidence is specific.

Where the evidence is strong. Structural and structural-adjacent family approaches are supported for child and adolescent behaviour problems, for substance misuse in adolescence (including multidimensional family therapy, which descends from the structural tradition), and for eating disorders in adapted forms. Minuchin's own work on anorexia and on psychosomatic families is historically important, though the early studies do not meet current methodological standards.

Where the evidence is moderate. Family-based interventions for delinquency and for school refusal, where structural elements are combined with behavioural and multisystemic components. The evidence rarely supports structural therapy alone; it supports structural work as a component of a multi-component approach.

Where the evidence is thin. Structural family therapy as a stand-alone treatment for adult individual presentations, for couple distress (where behavioural and emotion-focused models have better evidence), and for complex trauma.

What the matrix cell should say. "Moderate to strong for child and adolescent behaviour problems, substance misuse and eating disorders, usually as a component of a broader family-based programme; thin as a stand-alone treatment for adult and couple presentations." That cell is specific, it is honest, and it would survive a viva.

The practitioner's claim. "I am using structural work with this family because the maintenance pattern is organisational and I can act on it in the room, and because there is

evidence for family-based structural approaches with this kind of presentation — and I will measure the outcome in six weeks." The claim is modest, accurate and defensible, which is what the course is trying to teach.

4.11a Structural work with a family in which the father is absent by circumstance

The commonest structural mistake in a Nigerian clinic, and the correction.

The case. A family of five: mother (39, trader), three children (14, 11, 6), and a father who works in Abuja and returns every six to eight weeks. The referral is the 14-year-old's withdrawal and falling grades. The mother describes herself as "the only parent". The father, reached by telephone, is courteous, worried, and entirely absent from the household's daily operation.

The bad structural reading. "Peripheral father; weak parental subsystem; mother–adolescent enmeshment." The reading is accurate as a description and wrong as a formulation, because it treats a circumstance as a choice and implies that the father should be present, which he cannot be.

The better formulation. The household's executive subsystem is the mother, alone, for seven weeks in eight. The father holds authority without information: his decisions are made from a distance, on the basis of a summary, and they arrive as interruptions. The adolescent's withdrawal coincides with the father's last visit, at which a school decision was made without the mother's agreement. The structural problem is not the father's absence; it is the **uncoordinated decision-making across the two locations.**

The structural intervention, adapted.

First, make the two-location system explicit. Draw the map with the father in a separate box labelled "Abuja, six weeks", connected to the mother by a line labelled "telephone, Sundays" and to the children by a dotted line labelled "visits". The family corrects the map, and the corrections are the assessment.

Second, create a decision-making channel that works at a distance. A weekly fifteen-minute call between the parents, at a fixed time, whose purpose is decisions rather than reports, and at which the children are not present. The structural claim is that a parental subsystem can exist across two locations if it has a routine.

Third, use the visit as a structural event. The father's returns should be planned with the mother before he arrives: what will be decided, who will be told, and what the children will experience. A visit that is unplanned is a disruption; a visit that is planned is an executive meeting with affection in it.

Fourth, give the adolescent a legitimate position. The 14-year-old is currently the household's third adult — informed of the finances, consulted about the younger children, and therefore unable to be a child. The structural move is to return that responsibility to the mother and the aunt, explicitly, and to say why: "you have been carrying something that is not yours to carry."

What the model contributes here. Not a demand that the father be present, which would be an imposition of the model's norm, but a precise description of a two-location structure and a set of moves that make it work. The correction of the commonest error — reading a circumstance as a deficit — is the whole of culturally competent structural work.

4.12 Key terms

Term	Definition
Structure	The repetitive patterns of interaction that organise a family
Subsystem	A unit within the family defined by a boundary (couple, parental, sibling, individual)
Boundary	The rules governing participation, closeness and inclusion
Diffuse boundary / enmeshment	Too little separateness; participation and feeling shared without limit
Rigid boundary / disengagement	Too little contact; members isolated from one another's experience
Hierarchy	The distribution of authority and responsibility, especially between generations
Alignment	A joining of members for a purpose
Coalition	A joining of two or more members against a third
Cross-generational coalition	A coalition across the generational line, typically a parent and child against the other parent
Complementarity	The reciprocal relation between behaviours, so that over-functioning in one produces under-functioning in another
Identified patient	The member who carries the symptom, understood as the place where a structural problem becomes visible
Joining	The therapist's deliberate accommodation to a family's style and rules in order to be accepted
Enactment	Bringing a relationship into the room by instructing members to deal with each other directly
Unbalancing	Deliberately supporting one member to shift the system's equilibrium
Intensity	Raising the emotional temperature until the pattern cannot be avoided

4.13 Self-test

1. What is structure, and why is it visible in the session?
2. Distinguish a coalition from an alignment, and give a cross-generational example.
3. What is the difference between the couple subsystem and the parental subsystem?
4. Why must joining precede challenging?
5. Describe enactment, unbalancing, boundary-making and intensity, with one hazard for each.
6. What is the structural reading of the Adeyemi family, and what would falsify it?

7. What are the two elements of an honest evidence claim for this model?
8. Name three configurations in which the model's norm of hierarchy does not fit a family you might see.

Answers. 1. The repetitive interaction patterns that organise a family; because families enact them in the room — who speaks, who is interrupted, who sits where, who answers for whom. 2. An alignment is a joining for a purpose; a coalition is a joining against a third. Cross-generational example: a mother and daughter allied against the father's authority. 3. The couple subsystem concerns the partners' relationship; the parental subsystem concerns their joint executive authority over the children. A family can have a strong parental subsystem and a weak couple. 4. Because a challenge delivered without a join is experienced as an accusation, and the family will comply and hide or leave. 5. Enactment (hazard: begun too early, it produces a fight); unbalancing (hazard: it becomes siding); boundary-making (hazard: exclusion without explanation); intensity (hazard: it becomes the therapist's frustration expressed through technique). 6. Weak parental subsystem, mother–daughter cross-generational coalition, peripheral father who criticises when pulled in; falsified if a jointly agreed rule held for two weeks changes nothing in the mornings. 7. That the evidence is moderate to strong for child and adolescent behaviour problems, substance misuse and eating disorders, and thin as a stand-alone treatment for adult and couple presentations. 8. The three-generational household; the household divided by migration; the extended family as the executive; and any family where the father's authority is coercive.

4.13a The task, and the four tests of a good one

Structural work almost always ends with a task, and the task is where most structural therapy fails outside the room.

The four tests.

Test 1 — does it enact the structure rather than describe it? "Agree one sentence and both say it" enacts a parental subsystem. "Try to communicate better" describes an aspiration and enacts nothing.

Test 2 — is it specific enough to fail? A task that cannot fail cannot succeed either. "Have a conversation this week" cannot be evaluated; "on Wednesday, after the children are in bed, spend fifteen minutes on the school decision, and nobody leaves the room" can.

Test 3 — does it require the subsystem you are trying to strengthen? If the goal is a parental subsystem, the task must require the parents to act together, not each of them to act on the child.

Test 4 — can this family do it in this week? The task must fit the household's actual week — shifts, travel, school runs, the grandmother's presence. A task designed for a family with an empty evening will fail, and the failure will be read as resistance.

The commonest task failures. The task that is really advice ("try to be firmer"); the task that requires a person to feel something ("tell her how you feel"); the task that is too big to

start ("spend every evening together"); and the task that the therapist has not checked with the family ("I would like you to..." followed by silence and no question).

How to close a session with a task. State it; ask each member whether they can do it; ask what might get in the way; ask what they will do if it does; write it down; and give the family the note. Then, at the next session, ask about the task first, before anything else, because the family will read the therapist's attention as the measure of what matters.

4.14 Reflective task

Draw the structural map of your own family of origin: subsystems, boundaries, the hierarchy, and one coalition. Then write 250 words on which feature of the map you found hardest to draw, and why. Students usually find that the hardest feature is a boundary they have never described — a silence, an exclusion, an alliance nobody names — and that the difficulty of drawing it is the difficulty of working with it in someone else's family.

4.15 Readings for this week

- **Minuchin (1974)**, Families and Family Therapy, the whole of it, and twice for the transcripts.
- **Minuchin & Fishman (1981)**, Family Therapy Techniques, for enactment, unbalancing and intensity, demonstrated in dialogue.
- **Minuchin, Rosman & Baker (1978)**, Psychosomatic Families, for the historical application to anorexia.
- **Colapinto**, any review chapter on structural family therapy, for the systematic summary.
- **A recorded session by Minuchin**, watched twice: once for what he says, once for what he does with his body, his seating and his timing.

Next: Week 5 — Strategic and MRI brief therapy

Week 5 — Strategic and MRI brief therapy

The question. What is the family doing that keeps the problem going — especially their attempted solutions?

5.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The Palo Alto problem	Why a group of researchers stopped asking why and started asking what	Name the attempted solution in one case
10–25	Four core concepts	Attempted solution; function of the symptom; resistance as interaction; brevity	Trace the loop through a transcript
25–40	Reframing	A reframe built live, with rules	Draft three reframes for one behaviour
40–55	Directives and ordeals	Design and consent, demonstrated	Design one directive with a safety check

Minutes	Segment	What the lecturer does	What students do
55–70	Paradox, and its ethics	Prescribing the symptom: when, how, never	Decide three cases under supervision
70–85	The brief architecture	The five-session frame, termination by design	Plan a four-session contract
85–90	Critique and matrix row	Manipulation, evidence, depth	Fill the row

5.1a The Palo Alto context, in five sentences

A short historical note, because the school's assumptions are explained by the problem it was built to solve.

> In the 1950s and 1960s, a group of researchers at the Mental Research Institute in Palo Alto — Bateson, Jackson, Weakland, Watzlawick, Haley — studied communication in families, and concluded that a problem is not a thing inside a person but a pattern between people, maintained by what each does in response to the other. They noticed that families arrive having already tried to solve the problem, that the attempts are sensible, and that they are the reason the problem persists. They therefore stopped asking why a family has a problem and began asking what the family is doing that keeps it going, and they shortened therapy accordingly. The school's optimism — that a small, well-designed interruption can change a system quickly — is the direct product of that shift.

What the five sentences omit. The research programme's debt to cybernetics (MFT 701 Week 1), the disputes that split the group (Haley left to develop strategic therapy; the MRI group developed the brief model), and the fact that the school's founders were working with a clinic population for whom long therapy was unavailable.

Why the origin matters. The model's characteristic strength — precision about the present loop — and its characteristic weakness — thinness about history, trauma and grief — both follow from the problem it was designed to solve. That is true of every model in this course, and the week's matrix row should record it.

5.2 Core concepts

The strategic and MRI schools — associated with the Mental Research Institute at Palo Alto, with Jay Haley, John Weakland, Paul Watzlawick, Don Jackson and Cloe Madanes — produced the first genuinely brief family therapies, and their central idea is still the most economical explanation of why problems persist.

The problem is maintained by current interaction. Not by history, not by an internal object, not by a multigenerational pattern: by what is happening now, in the loop between the members. The past explains how the loop began; it does not maintain it, and changing it does not require working through the past.

The attempted solution maintains the problem. This is the school's signature idea, and it is learnt best by example. A mother who wakes her son five times each morning to

get him to school is solving the problem of his not getting up; her solution ensures that he never has to get up himself, and the problem persists. A couple who avoid discussing money in order to prevent arguments have a marriage in which money cannot be discussed, and the avoidance is now a problem in its own right. The attempted solution is usually sensible, kindly and nearly effective, which is why it continues.

The function of the symptom. The symptom may serve a purpose in the system: it may prevent a worse conflict, keep a person at home, discharge an obligation, or hold a family together. The concept is powerful and speculative; it is the place where the school has most often overreached, and students should be able to state both the insight and the overreach.

Resistance is a response to the therapist, not a property of the family. If the family does not follow a task, the therapist's formulation, framing or timing was wrong. This is a refreshing idea after models that describe families as "resistant", and it has an obvious failure mode: it can leave the therapist endlessly responsible for everything that goes wrong.

Brevity by design. Therapy is short not because families are undeserving of long work but because a focused intervention on a maintained loop can produce change quickly, and because a short contract sets an expectation of change. The school's practitioners often set a limit of five or ten sessions at the outset.

First-order and second-order change. Watzlawick's distinction: first-order change alters behaviour within a system that stays the same (more of the same, or less of it); second-order change alters the system's rules, so that the behaviour that was a solution is no longer necessary. The distinction is the school's theory of change, and the interventions are designed to produce the second.

5.2a Six attempted solutions that recur

The concept is easy to understand and hard to spot in a live session. Six patterns that recur across presentations, with the theme that unites them.

Pattern 1 — the repetition of the request. The parent asks, asks again, raises the volume, and asks a fifth time. The theme: responsibility is carried by the asker. The child learns that the request can be waited out, and the parent learns that only escalation works.

Pattern 2 — the protective explanation. The family explains the absence, the failure, the behaviour, to the school, the employer or the relatives. The theme: the consequence is absorbed by the family. The member never meets the consequence of their own decision.

Pattern 3 — the supervision of the incapable. One member is treated as unable to manage a task, and another accompanies, reminds, checks and completes it. The theme: competence is not permitted to be tested. The pattern is always kindly meant, and it prevents the discovery that the member could manage.

Pattern 4 — the reconciliation that prevents resolution. After each conflict, one member makes peace — a meal, a gift, a joke, a silence — before the disagreement has

been resolved. The theme: the conflict is ended by the person who cannot bear it. The disagreement returns, unaddressed, and the peacemaking becomes part of the cycle.

Pattern 5 — the avoidance of the subject. The family does not discuss money, the father's drink, the daughter's boyfriend, the grandmother's illness. The theme: the topic is too dangerous to name. The avoidance protects the family from a conflict and ensures that the topic governs the household from a distance.

Pattern 6 — the increase in surveillance. A parent who is worried checks the telephone, the school bag, the friends, the whereabouts. The theme: trust is replaced by control. The adolescent responds by concealing, which increases the worry, which increases the surveillance.

The exercise. For each pattern, write the one-sentence reframe and the one-sentence directive that would follow. The reframe names the theme without blame ("the two of you have arranged it so that one carries the asking and one carries the waiting"); the directive interrupts it ("for one week, ask once, and then write down what happens").

What the exercise teaches. That the attempted solution is usually visible in the first ten minutes, in the way the family describes what they have already done. A therapist who listens for the theme rather than for the story will have a formulation by the end of the first session, which is what the model requires.

5.3 Therapy: the active strategist

The therapist in this model does four things, in order, and each is a designed move rather than a conversational development.

Step 1 — define the problem in behavioural terms, and find who is affected. A specific behaviour, in a specific context, with a specific person doing it. "He is lazy" is not a problem definition; "he does not get out of bed before the fifth request, on school days, and his mother then dresses him" is.

Step 2 — trace the attempted solutions and their persistence. What has the family done about it, in what order, and what happened? The list is usually long, and the interventions are usually variations on one theme. The theme is the target.

Step 3 — find the smallest intervention that interrupts the loop. The school's preference is for the smallest sufficient move, delivered as a task, with a clear rationale. Big interventions are reserved for entrenched loops.

Step 4 — set a limit and review. A stated number of sessions, an explicit review, and termination by design rather than by drift. The limit is itself an intervention: it tells the family that change is expected soon.

The stance. Active, pragmatic, often directive, and deliberately focused on the present. The therapist takes responsibility for the design of change, and the family takes responsibility for carrying it out. This allocation is the source of the model's efficiency and of its characteristic ethical risk (5.6).

What the therapist deliberately does not do. Does not explore the family's history beyond what is needed to understand the loop; does not interpret the family's dynamics for insight; does not wait for the family to arrive at a plan; and does not treat the family's reluctance as a topic in itself.

5.3a Listening for the attempted solution

A practical exercise that teaches the model's core perceptual skill in fifteen minutes, done in pairs with a script.

The script. One partner reads a four-paragraph account of a family problem, in the family's voice, without interruption. The account contains the presenting complaint, the history, and — buried in the middle — a list of what the family has done. The listener takes notes on one question only: what have they already tried, and what is the theme?

A sample account, condensed. "Our son is 13 and will not do his homework. He says the teachers do not explain. We have spoken to the school twice. My husband sits with him every evening, which ends in shouting. I have tried taking the games console away, but he then does nothing at all and the teachers complain. We have tried a tutor, which he liked for three weeks. We have tried a reward chart. Last term his report was poor and I cried in front of him, which I regret. He is not a bad boy. His father says I make excuses for him."

The attempted solutions, extracted. Speaking to the school twice; the father sitting with him every evening; removing the console; a tutor; a reward chart; crying in front of him. **The theme:** every attempt is an increase in supervision and pressure, applied by an adult, addressed to the boy's behaviour rather than to the task. The boy's response is to do nothing.

The reframe that follows. "You have tried everything that can be done to him — sitting with him, removing things, paying a tutor, talking to the school. He has learned that the homework is something adults do with him or about him. What would it look like if the homework were something he had to fail at, once, in a way you did not rescue?"

The directive that follows. "For two weeks, nobody sits with him. He does the work or he does not. You will not speak to the school about homework, and you will not remove anything. You will keep a note of what he says at breakfast about the day before." The design interrupts the theme (supervision and pressure), which is what makes it a strategic intervention rather than a suggestion.

The debrief question. "Could you have written the theme without the notes?" Most trainees cannot on the first attempt, because they listen to the story rather than to the list of attempts. The exercise is repeated weekly, with a different script, until the listener can produce the theme in one sentence on first hearing. That ability — to hear the attempted solutions inside a family's narrative — is the whole of the model's assessment, and it is a trainable skill rather than a talent.

5.4 Signature techniques

Reframing. Offering a new frame for the same facts, such that a different response becomes possible. The example from the course material: "Your daughter's tantrums may be her way of keeping you two in the same room." The reframe is not a reassurance; it is a claim about the function of the behaviour, offered as a possibility, and its purpose is to change what the parents do rather than to make them feel better.

The three rules of a good reframe. It must be plausible (the family can entertain it); it must be in the family's own language and values (a reframe that imports the therapist's values will be rejected); and it must point to a different action (a reframe that produces only a feeling is a compliment).

Directives. A task for the week, designed to interrupt the loop. Directives are of two kinds.

Straightforward directives instruct the family to do something different: "for one week, do not wake him more than twice; if he is not up, let him be late, and do not write a note for the school." The design requires that the family can do it, that the consequence is survivable, and that the therapist has thought through what will happen.

Indirect directives are the paradoxical family (5.5), and they require supervision.

Ordeals. A task that must be performed whenever the symptom occurs, making the symptom more costly than the change. Haley's classic example is a task that must be completed before the symptom is allowed to happen. Ordeals are legitimate where the symptom is voluntary and low-risk (a bedtime ritual, a complaint, a habit) and are never legitimate where the symptom is a compulsion, an addiction with medical risk, or a self-harm behaviour.

The pretend technique. "For one week, pretend to have the symptom on purpose, for ten minutes a night, at a time you choose." The technique changes the symptom from something that happens to the family into something the family controls, and it produces information: a symptom that cannot be produced on demand is a different kind of symptom.

Prediction and observation tasks. "Between now and next week, notice the times when the problem does not happen, and write down what is different." These tasks gather data, change attention, and are almost always safe — which makes them the right first task for a trainee.

5.4a A reframe, built in four steps

Reframing is the model's most portable technique and the one trainees most often mistake for a compliment. Four steps, worked on one behaviour.

The behaviour. A 15-year-old has been stealing small amounts of money from her mother's purse.

Step 1 — get the family's meaning. "What do you think it means when she takes the money?" The mother's answer: "she has no respect." The meaning is moral, and it is the frame that must be changed.

Step 2 — find the alternative meaning that is both plausible and useful. Look for a function: what does the theft do, or what does it express? In this case, the girl has no money of her own, her friends have money, and she has never asked for any. The alternative meaning: the theft is a way of getting something she cannot ask for.

Step 3 — offer it in the family's own values, tentatively. "In this family, asking for money for yourself may be harder than taking it. I wonder whether what looks like disrespect is actually a girl who cannot say 'I need something'." Note the three elements: the family's values ("in this family"), the tentative grammar ("I wonder"), and the link to a different action (asking).

Step 4 — attach an action. A reframe that produces no change of behaviour is a sentiment. The action here: "for two weeks, she has a set amount each week, given without comment, and she does not have to explain it. If she takes anything else, you will know that it is not about the money."

What makes the reframe work. It does not deny the theft, does not excuse it, and does not require the mother to abandon her values. It changes the behaviour from a moral fact into a communication, which makes a different response available. The mother can now do something other than police and punish.

What would make it a bad reframe. "She is stealing because she needs your love" — implausible, sentimental, and importing the therapist's theory. "She is stealing because teenagers do that" — dismissive of the mother's concern. "She is stealing because you work too much" — blaming, and it points to no action the mother can take.

The test. A good reframe produces a change in what the family does in the next seven days. If the family says "that is interesting" and nothing changes, the reframe was a compliment.

5.5 Paradox, and its ethics

Prescribing the symptom is the school's most controversial technique and its most misused. It deserves a full section.

What it is. The therapist instructs the family to continue, or deliberately to produce, the behaviour they want to stop — with a rationale, for a limited time, and with careful framing.

Why it can work. Three mechanisms, none fully established. It removes the struggle: when the therapist stops pushing for change, the family's resistance to change disappears with the pressure. It places the behaviour under voluntary control, which changes the family's relationship to it. And it exposes the system: when the family is asked to produce the symptom deliberately, the members discover what the symptom does for them.

When it is defensible. Where the loop is entrenched and the family's attempted solutions have all failed; where the symptom is voluntary and low-risk; where the therapist has a strong join and the family's trust; where there is supervision; and where the therapist has a plan for what happens if the symptom escalates.

When it is indefensible. Where there is any risk of harm — self-harm, violence, substance use with medical risk, an eating disorder, a psychosis; where the family has not consented to the frame and would feel tricked if they understood it; where the therapist is working alone without supervision; and where the therapist's motive is to demonstrate cleverness.

The consent question, stated plainly. A paradoxical intervention works partly by not being fully explained. The ethical position is that the family must be able to understand the frame if they think about it, must not be deceived about anything material, and must be able to refuse. "I am going to ask you to do something that will seem strange, and I will explain why afterwards" is honest; a directive whose rationale the therapist conceals is not.

> **CAUTION**
> No trainee in this course should use a paradoxical directive without prior discussion in supervision and a documented rationale. The technique's literature is thin, its failure modes are serious, and its reputation has damaged the field.

5.6 The brief architecture: a five-session frame

The model is practised as a short, focused therapy with a defined end. Here is a workable structure.

Session 1 — define, trace, reframe, task. Ninety minutes or a standard hour. Define the problem behaviourally, trace the attempted solutions, offer one reframe, and give one task. Book the next session and state the total number of sessions you propose.

Session 2 — review the task, refine, second task. The task is the first agenda item. What happened, what did the family notice, what got in the way? Then adjust the design and give the second task, which should be a development of the first rather than a replacement.

Session 3 — the loop in the room. By now the maintain pattern should be visible in the session. Work with it directly: an enactment, a reframe delivered in the moment, or a directive designed from what has just happened.

Session 4 — consolidate. What has changed, what has not, what would maintain the change? The family is asked to describe the difference in their own words, which is the school's version of the MFT 701 Week 14 ending review.

Session 5 — review and finish. The outcome, measured if possible; the plan if the problem returns; the ending, named. A sixth session is added only if the review identifies a specific, time-limited piece of work, and the reason is recorded.

Why the architecture works. It front-loads the definition and the first task, which is where brief therapy earns its brevity; it uses the second session to correct the therapist's errors; and it makes the ending a design feature rather than a failure.

The risk of the architecture. It can become a formula applied to families who need something else — a family in which the presenting problem is the tip of a trauma, a violence, or a bereavement. The assessment question is not "how many sessions?" but "is the maintaining pattern accessible to a focused intervention?" and the honest answer is sometimes no.

5.6a The five-session frame, worked through one case

The architecture is easier to teach through a single case carried from session one to session five, with the therapist's decision at each point.

The case. A couple, married six years, two children (4 and 2). The presenting complaint: "we never finish an argument; one of us walks out." The wife walks out; the husband follows her from room to room.

Session 1 — define, trace, reframe, task. Definition: on most weekends, an argument begins about the children's bedtime; the wife leaves the room; the husband follows; the argument continues in the corridor and ends unresolved; both are exhausted by Sunday night. Traced solutions: the wife leaving (to end it), the husband following (to finish it), a rule that they will not argue in front of the children (which produces the corridor), and going to bed separately (which ends the evening). Reframe: "You are both trying to end the argument, and you are using opposite methods. One of you ends it by leaving, one by staying. The corridor is where the two methods meet." Task: for one week, when the argument begins, the wife stays in the room for two minutes without speaking, and the husband does not move towards her. Two minutes, timed.

Session 2 — review and refine. The task produced one completed two-minute silence, and one failure. The failure is more informative: the husband moved, the wife left, and both reported the same feeling — "if I stay still, something worse will happen". The refinement: the two minutes are now spent with a stated topic ("the bedtime plan"), so that silence is not required, only the absence of leaving.

Session 3 — the loop in the room. The therapist asks them to discuss the bedtime plan for five minutes without leaving their chairs. The pattern appears within ninety seconds: the wife leans back, the husband leans forward. The therapist names the movement, which the couple had never noticed, and asks what each was feeling at the moment of the lean. The answer, from both, is fear of the other's disappearance.

Session 4 — consolidate. Both report that the corridor has not been used for two weeks. The therapist asks them to explain the change to each other, which produces a description the couple can own. A second task is agreed: one conversation a week about the children's week, on a stated evening, for twenty minutes.

Session 5 — review and finish. The measure (a simple weekly rating of "how much of the weekend was spent arguing") has fallen from 8 to 3. The therapist names the ending, reviews what changed, and writes down what the couple will do if the pattern returns.

What the case demonstrates. The model's power is in the first and third sessions: a definition precise enough to work with, and a loop made visible in the room. Sessions two, four and five are consolidation, and they could be compressed into one if the service

required it. The case also demonstrates the model's limit: the couple's fear of the other's disappearance is an attachment issue that this formulation named but did not treat, and a longer therapy would have taken it up.

5.7 The Adeyemi loop, traced

An application that shows the model's method on the running case.

The behavioural definition. On school days, Amara does not get out of bed before the fourth or fifth request; the mother then telephones the school with an explanation; Amara stays at home; the mother is angry and guilty for the rest of the day.

The attempted solutions, in order. Asking; then insisting; then shouting; then removing the telephone; then telephoning the school; then taking leave from work to supervise the mornings; then involving the school counsellor; then prayer and fasting. Each was sensible; each was a variation on one theme: the mother takes responsibility for the morning.

The loop. The mother's responsibility removes the consequence from Amara; the absence of consequence makes the refusal costless; the refusal increases the mother's anxiety; the anxiety increases her responsibility; and the morning repeats with less hope and more resentment.

The reframe. "You have been protecting her from the consequences of her own decision, because you are frightened of what the school will do and of what people will think. Protecting her has meant carrying the whole morning yourself. That is not a failure of love; it is love in a form that has stopped working."

The directive. "For two weeks, you will make one request at six o'clock, and a second at half past. If she is not up by seven, you will go to work. You will not telephone the school. You will let her be absent without an explanation, and you will let the school's letter arrive." The design includes the consequences the mother fears, which is why the reframe comes first and why the therapist must have discussed the school's reaction with her.

What the model sees and what it misses. It sees the loop precisely and it produces a task that could change it within a fortnight. It does not see the mother's grief, the father's absence, the grandmother's death, or the meaning of school for Amara — and a task delivered without those would be cruel. The mature use of this model is therefore almost always in combination, which is the subject of Week 12.

5.7a The matrix row, written

Dimension	Strategic / MRI brief therapy
Theory of problem	The problem is maintained by current interaction, especially the family's attempted solutions, which are sensible, nearly effective and therefore repeated
Unit of focus	The behavioural loop between the members, in the present, around a specific problem
Theory of change	Interrupt the loop, usually with a task, so that second-order change becomes possible; insight is not required

Dimension	Strategic / MRI brief therapy
Therapist position	Active strategist who designs interventions, takes responsibility for the design, and sets a limit
Signature techniques	Reframing; straightforward directives; ordeals; the pretend technique; prediction and observation tasks; paradoxical directives under supervision
Evidence base	Moderate for brief strategic family approaches in adolescent behaviour problems and substance misuse; strong component evidence (goals, tasks, brief contracts); paradoxical techniques unevidenced
Cultural assumptions	That the therapist has a mandate to direct; that a behavioural task is a legitimate form of help; that brevity is a virtue
Blind spot	Can be manipulative; can be superficial where trauma, grief or severe mental illness is present; the function of the symptom is speculative; may fit a family's problem to the service's budget

The viva sentence. "I use this model where the maintaining pattern is behavioural and the family wants something practical, I design the smallest intervention that interrupts the loop, I hold the ethics of direction explicitly, and I review at session four."

5.8 Annotated transcript: building a directive with a reluctant father

A ten-turn sequence from a second session with the Okonkwos. The task is not delivered; it is negotiated, which is the skill this transcript exists to teach.

> **T1 Therapist:** Last week I asked you both to notice the mornings. What did you notice?
> **T2 Mother:** I noticed I do it all. Five times. And on Saturday he did it once and Ebuka got up.
> **T3 Therapist:** Once. And it worked.
> **T4 Mother:** It worked for him.
> **T5 Therapist:** Mr Okonkwo, what did you do that was different?
> **T6 Father:** I do not know. I said, "Ebuka, I am going to work" and I went downstairs.
> **T7 Therapist:** And he got up.
> **T8 Father:** He got up. (pause) Because he knows I will leave.
> **T9 Therapist:** That is the whole task, and you did it by accident. I want to ask you both to do it on purpose, and I want to ask you something difficult: for two weeks, the morning belongs to your husband. Mrs Okonkwo, you will stay in bed until seven. Not because you are lazy — because the two of you have arranged it so that you carry a responsibility that is not only yours.

> **T10 Mother:** (long pause) If I stay in bed, and she is late —
> **T11 Therapist:** Then she is late, and the school telephones your husband.

Annotations. T1 makes the task the first agenda item, which is the model's discipline: the family learns that the therapist's attention is on what happens between sessions.

T2–T8 is the discovery of the intervention inside the family's own material. The therapist did not design a task from theory; she asked what happened and found that the father's

ordinary behaviour was already the intervention. This is the school's method at its best, and it is far more persuasive to a family than an imported technique.

T8 is the father's own explanation, and the therapist's T9 uses it verbatim. The phrase "the morning belongs to your husband" is the task in four words, and it is a structural idea delivered as a brief-therapy directive — a small demonstration that the models overlap in practice more than they do in their literature.

T9 also does the reframe's work in the same breath: "not because you are lazy — because the two of you have arranged it". The mother is not blamed, the arrangement is named, and the task follows from the naming.

T10 is the objection, and it is the moment most trainees lose the directive: they reassure ("it will be fine"), or they withdraw the task ("perhaps we start with one day"). T11 instead completes the design by naming the consequence, which is what the mother needs in order to be able to agree.

What the therapist did not do. Did not explain the theory; did not say "this is a strategic intervention"; did not ask the father whether he was willing (she asked the mother, who held the obstructing behaviour); and did not pretend the task was easy.

The follow-up, two weeks later. The father reported three mornings of five in which Ebuka was up before seven. The mother reported that she had stayed in bed for four mornings and had telephoned the school once. The therapist's question was not "did it work?" but "what did you learn about the arrangement?" — which is the model's version of consolidation, and the bridge to the next task.

5.9 Laboratory: design one directive

The week's practical, ninety minutes, and the assessment's rehearsal.

Stage 1 (15 minutes) — the case. Each student brings a disguised family from placement or the case bank, with the problem defined behaviourally and the attempted solutions listed. Pairs exchange cases.

Stage 2 (25 minutes) — the design. For the partner's case, design one directive that makes the current attempted solution impossible to continue, without instructing the family to do the opposite in so many words. The design must state: the task in one sentence, the rationale the family will be given, the safety check, the likely objection, and the response to the objection.

Stage 3 (25 minutes) — delivery. Deliver the directive to the partner, who plays the family and objects at least twice. The observer scores against the rubric.

Stage 4 (25 minutes) — the group judgment. Each directive is presented to the group, which judges three things only: is it clear? is it doable in this family's week? is it safe? A directive that fails any of the three is redesigned in front of the group.

Marking rubric.

Criterion	Weight	Full marks
Definition	15%	The problem is behavioural, with a context, a person and a consequence
Loop analysis	20%	The attempted solutions are traced and their common theme named
Design	25%	The directive interrupts the theme; is specific; and is achievable in the family's actual week
Delivery	20%	One sentence, clear rationale, both objections handled without withdrawal or reassurance
Safety	20%	The risks named, the consequences thought through, and no risk of harm to any member

The commonest failure. A directive that requires the family to feel something rather than to do something. "Tell her how you feel when she shouts" is not a strategic directive; it is an invitation, and the model's power lies in behaviour that can be observed and reviewed.

5.9a Three directives that failed, and why

The failures teach the design better than the successes.

Failure 1 — the task nobody could do. A father on night shifts was asked to have breakfast with his son every morning for two weeks. He managed twice. The task failed because it was designed for a household with a different week. The correction: the therapist's question before designing any task — "walk me through your week, hour by hour" — is not optional, and it is the single most common omission in trainee designs.

Failure 2 — the task that required a feeling. A wife was asked to "let him see that you are hurt rather than angry". She reported that she had tried and that "it came out as anger again". The task failed because it asked for an internal state rather than an observable behaviour. The correction: convert the feeling into a behaviour — "say one sentence: 'I am hurt, not angry' — whether or not it sounds true" — which is doable, observable and reviewable.

Failure 3 — the task that took sides. A mother was asked to stop defending her son when the father criticised him. The father was delighted; the mother felt accused; the son concluded that the therapist had joined the father. The correction: the task was correct in its analysis and wrong in its distribution. A better design requires both parents to change something, so that neither is the identified problem: "when either of you criticises, the other will ask one question before responding."

What the three failures have in common. Each was designed from a correct formulation and without sufficient attention to the family's week, the observability of the behaviour, or the distribution of the change. Those three considerations are the design checklist, and they are worth writing on the whiteboard during the laboratory.

The professional habit. Before delivering any task, the therapist says it aloud and asks three questions silently: can this family do it in this week? Can I tell on Tuesday whether they did it? Does it require everyone to change something, or does it identify one person

as the problem? A task that fails any of the three is redesigned in the session, in front of the family, which is itself a demonstration of the model's pragmatism.

5.10 Six errors in strategic work

1. **The clever intervention.** A design that impresses the therapist and confuses the family. Fix: the family must be able to say the task back in one sentence; if they cannot, it is too clever.
2. **The unearned paradox.** Prescribing a symptom without a join, supervision or a safety analysis. Fix: no paradox without supervision and a documented rationale, as a course rule.
3. **The directive without consequences.** A task whose failure has no consequence, which therefore cannot be evaluated. Fix: build the consequence in, as T11 did in 5.8.
4. **The missing function.** Treating the symptom as pure nuisance when it is doing something for the family. Fix: ask what would be lost if the symptom stopped, and listen to the answer.
5. **Brevity as a budget decision.** Ending at five sessions because the service allows five. Fix: the limit is a clinical decision, stated to the family with a rationale, and reviewable.
6. **The therapist's responsibility for everything.** Using "resistance is my problem" to take on responsibility for every failure, including the family's choices. Fix: the model allocates responsibility; state the allocation aloud and hold it.

5.11 Culture, faith and the strategic stance

The model's cultural profile is distinctive: it is unusually portable in some respects and unusually risky in others.

Where it travels well. The model's core question — what is the family doing that keeps this going? — requires no cultural vocabulary, and its interventions are behavioural, brief and practical. In services with high demand and limited sessions, in families who want a solution rather than an exploration, and in settings where a directive from a professional is expected and welcome, the model fits.

Where it is risky. Three features.

The directive assumes a mandate. A therapist who tells a family to change the morning routine is exercising authority. In families where the therapist's authority is contested, or where the therapist is young, female, or from a different ethnic or religious community, the directive may be heard as impertinence. The model's answer is joining, and the model understates how much joining a directive requires.

The reframe imports values. "Your daughter's tantrums may be her way of keeping you two in the same room" is a systemic reframe, and it is also a claim that the child's behaviour is about the parents. A family whose account is that the child is spiritually attacked, or that the behaviour is disobedience, will hear the reframe as a reinterpretation of their whole world. The reframe must be built from the family's own values: "in this

family, children hold the peace between adults; that is a heavy thing for a nine-year-old to carry" performs the same function in the family's register.

The function of the symptom can be a spiritual claim in disguise. The idea that a symptom serves the system is close to the idea that a symptom has a meaning or a purpose, which many families hold in a theological form. The therapist should be alert to the congruence and to its risks: a family may hear "she is doing it on purpose", which is the opposite of the model's claim.

The spiritual frame as a resource. Where a family's account is spiritual, a directive can be designed within it: a prayer to be said at a specific time, a fast undertaken with a stated purpose, a conversation with the pastor to be arranged by the father. The model's method is agnostic about content; it asks only whether the intervention interrupts the loop.

> **DEBATE**
> "Brief therapy is the model most suited to under-resourced services and the model most likely to do harm in them." Argue both sides.

5.11a Where the strategic and the Milan schools part company

The two schools are often treated as one "systemic" tradition. They are distinct, and the distinction is examinable.

On the therapist's knowledge. The strategic therapist acts as though they know what maintains the problem and designs accordingly. The Milan therapist proceeds as though the family's premises are unknown and uses hypothesising to discover them, treating the team's certainty as the thing to be disturbed.

On the direction of intervention. The strategic school intervenes in behaviour; the Milan school intervenes in meaning, using questions and, originally, long intervals between sessions.

On paradox. Both use it, and differently. The strategic paradox prescribes the symptom to interrupt a loop. The Milan positive connotation attributes a benign motive to the whole family system, and the intervention is the reframe rather than a task.

On the family's role. The strategic therapist gives the family a task to carry out. The Milan therapist gives the family a new description to think about, and expects the thinking to do the work.

On brevity. Both are brief by the standards of the era, and the strategic school is more aggressively so; Milan sessions were sometimes spaced by a month precisely because the intervention was cognitive.

Why the distinction matters for the matrix. A student who fills both rows with "systemic, task-based, brief" has not distinguished them and will lose marks. The distinguishing column is the theory of change: behaviour, in one case; premise, in the other.

5.12 Evidence note: what the trials support

The evidence for this school is better than its reputation among depth-oriented clinicians and worse than its practitioners' confidence.

What is well supported. Brief strategic and structural-strategic family therapy has trial evidence in adolescent behaviour problems and substance misuse — notably brief strategic family therapy for Hispanic adolescents in the United States, where trials showed reductions in substance use and behaviour problems. Problem-solving and brief family interventions have evidence in depression and in some anxiety presentations. The components — clear behavioural goals, between-session tasks, reframing, brief contracts — are supported across models and are now standard in most evidence-based protocols.

What is not supported. Paradoxical interventions have very little trial evidence, and their use rests on case literature and clinical argument. The "function of the symptom" is a clinical heuristic, not an established mechanism. Ordeals have essentially no evidence base and are best understood as a historical technique with a narrow, low-risk application.

What the matrix cell should say. "Moderate evidence for brief strategic family approaches in adolescent behaviour problems and substance misuse; component evidence strong (goals, tasks, brief contracts); paradoxical techniques unevidenced and to be used only under supervision."

The practitioner's claim. "I am offering a focused intervention because the maintaining pattern is behavioural and the family wants something practical, I will review at session four, and I will be honest that the techniques I use are supported mainly by their components rather than by trials of the package." That sentence is the standard for this week, and it is defensible in the examination.

5.12a The ethics of direction, stated as five rules

This model exercises more direct authority over families than any other in the first semester. Five rules make that authority defensible.

Rule 1 — say what you are doing, and why. "I am going to suggest something that may seem strange" is not a disclosure of technique; it is honesty about the fact that a technique is being used. Families are entitled to know that they are in a designed intervention rather than a conversation.

Rule 2 — never design a task the family cannot refuse. A refusal is information about the design, and a family that cannot refuse is being managed rather than treated. Ask, every time: "can you do this, or would something else fit better?"

Rule 3 — the risk of harm overrides the elegance of the design. No task that could produce violence, eviction, job loss, school exclusion or a medical risk, however neat the loop analysis. Where the consequence is serious, the design must be discussed with the family and, where relevant, with the school or employer in advance.

Rule 4 — paradox requires supervision, a rationale and a limit. Stated in 5.5 and repeated here as a course rule, because this is where trainees most often overreach. A paradoxical directive is documented, time-limited, and reviewed.

Rule 5 — the family's account of the outcome is the outcome. A task that produced a change the therapist wanted and the family did not want is not a success. Ask, at the review: "what changed, and was it the change you wanted?" The answer frequently revises the formulation.

Why five rules rather than a principle. Because the model's ethical risk is not a matter of intention but of practice, and practices are regulated by specific rules. A therapist who can recite the five rules, and who can say which one a given design tests, is safer than a therapist who is merely well-meaning.

The connection to MFT 701 Week 12. The confidentiality, competence and record-keeping obligations apply in full. The additional obligation of this model is transparency about direction, and it is the one that the model's own history makes necessary.

5.13 Key terms

Term	Definition
Attempted solution	The family's reasonable response that has become part of the problem
First-order change	Change within the system's rules, which leaves the system the same
Second-order change	Change of the system's rules, so that the attempted solution is no longer needed
Function of the symptom	The purpose the symptom serves in the system — a heuristic, not an established mechanism
Reframe	A new frame for the same facts that makes a different response possible
Directive	A between-session task designed to interrupt the maintaining loop
Ordeal	A task performed whenever the symptom occurs, making the symptom costly
Paradoxical directive	An instruction to continue or produce the symptom, for a limited time and with a rationale
Pretend technique	A task asking the family to produce the symptom deliberately for a set period
Resistance (strategic)	A response to the therapist's approach rather than a property of the family
Brief contract	A stated number of sessions with a review and an end by design
Positive connotation	A Milan descendant of reframing: attributing a benign purpose to the family's behaviour

5.14 Self-test

1. What is an attempted solution, and give two examples?
2. Distinguish first-order from second-order change.
3. What are the three rules of a good reframe?
4. When is a paradoxical directive defensible, and when is it not?
5. What is the difference between a directive and an ordeal, and what makes an ordeal unsafe?

6. Give the five-session architecture and the purpose of each session.
7. Trace the Adeyemi loop and state the directive that follows.
8. What does the evidence support, and what does it not?

Answers. 1. A reasonable response that maintains the problem: a mother who wakes a child five times, ensuring he never has to get up; a couple who avoid discussing money to prevent arguments, so that money cannot be discussed. 2. First-order changes behaviour within the system's rules; second-order changes the rules so that the attempted solution is no longer needed. 3. Plausible; in the family's own language and values; pointing to a different action. 4. Defensible where the loop is entrenched, the symptom voluntary and low-risk, the join strong, supervision available, and a plan exists for escalation; indefensible where there is risk of harm, no consent, no supervision, or the therapist's vanity is the motive. 5. A directive instructs a different behaviour; an ordeal requires a task whenever the symptom occurs. An ordeal is unsafe where the symptom is a compulsion, an addiction with medical risk, or self-harm. 6. Session 1 define, trace, reframe, task; session 2 review and refine; session 3 work the loop in the room; session 4 consolidate; session 5 review and finish. 7. The mother's responsibility removes the consequence, the refusal becomes costless, her anxiety rises, and the loop tightens; the directive is one request at six and one at half past, then go to work, and do not telephone the school. 8. Moderate evidence for brief strategic family approaches in adolescent behaviour problems and substance misuse, strong component evidence, and essentially no trial evidence for paradoxical techniques or ordeals.

5.15 Reflective task

Think of a difficulty in your own life that you have tried to solve more than three times, in more than three ways, with the same result. Name the theme of your attempted solutions in one sentence, and then design the smallest intervention that would break the theme — for yourself, not for anyone else. Most students find this exercise uncomfortable and productive in equal measure, because the model's central insight is easier to apply to a family than to oneself.

5.16 Readings for this week

- **Watzlawick, Weakland & Fisch (1974)**, Change: Principles of Problem Formation and Problem Resolution, the foundational text, and short.
- **Haley (1976)**, Problem-Solving Therapy, for directives and the strategic stance.
- **Haley (1980)**, Leaving Home, for the architecture of brief work with young adults.
- **Madanes (1981)**, Strategic Family Therapy, for the techniques with cases.
- **Watzlawick, Beavin & Jackson (1967)**, Pragmatics of Human Communication, chapters 5–6, for the theory of punctuation that underlies this week.
- **Any controlled trial of brief strategic family therapy**, read with the six questions from MFT 701 Week 11.

Next: Week 6 — Milan systemic therapy

Week 6 — Milan systemic therapy

The question. How do the family's beliefs and premises organise their behaviour — and how can curiosity disturb them?

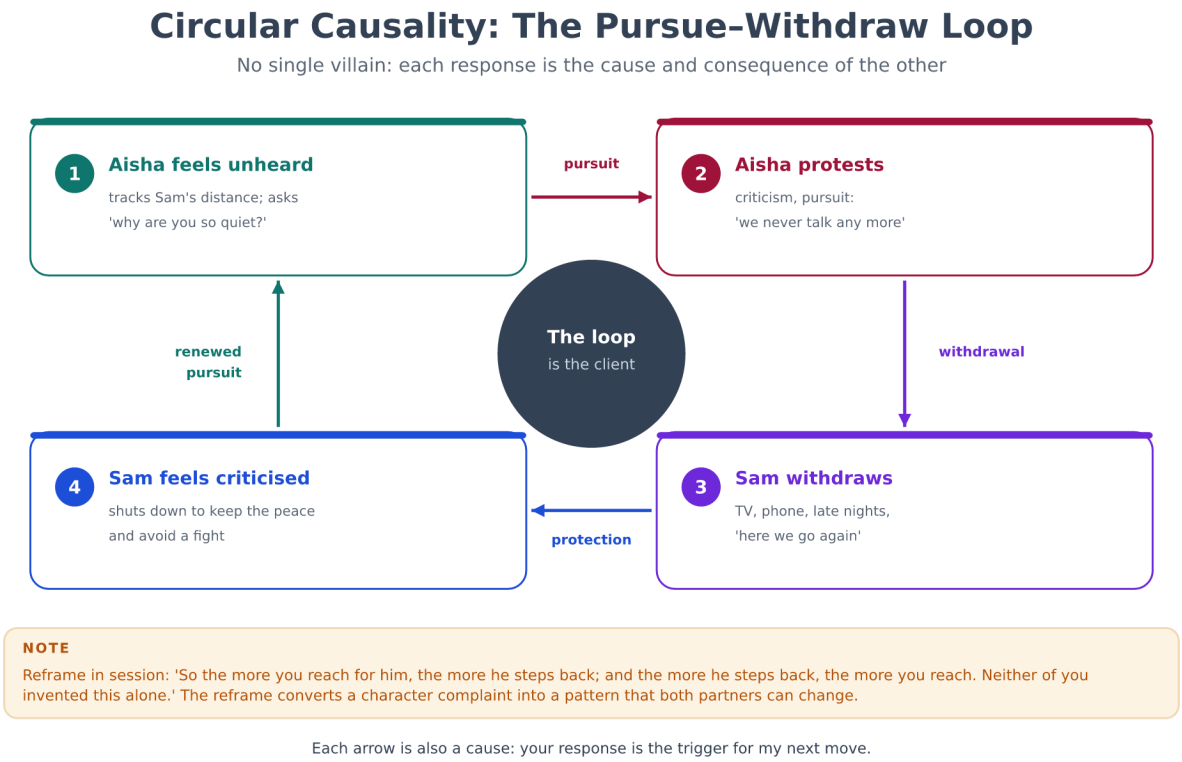


Figure 6.1 — Circular causality in practice

6.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	From Palo Alto to Milan	What the Milan team changed, and why it mattered	Name the difference from Week 5
10–25	Premises and the family belief system	Two families, one premise each	State the premise in a transcript
25–40	Hypothesising, circularity, neutrality	The three-part stance, defined precisely	Write three hypotheses for one family
40–60	Circular questioning	Six types, demonstrated	Ask only circular questions for ten minutes
60–72	Positive connotation	The logic and the hazard	Reframe three behaviours as system-serving
72–85	The reflecting team	The modern, non-directive version	Reflect in a fishbowl
85–90	Critique and matrix row	Neutrality in violence; cost	Fill the row

6.2 Core concepts

The Milan school — Selvini Palazzoli, Boscolo, Cecchin and Prata, working in Milan from the 1960s — took the Palo Alto insight about loops and added an account of meaning. The result is the most epistemically sophisticated model in this course, and the one whose central skill has spread furthest beyond its own practitioners.

The family as a meaning-making system. A family is organised not only by behaviour but by the premises it holds: unexamined assumptions about what a family is, what a man or a woman owes, what love looks like, what must never be said. The behaviour follows the premise. A family whose premise is "conflict destroys love" will produce a household in which nothing can be discussed; the silence is not a failure of communication, it is the premise being obeyed.

Premises are shared, and they are held by the helpers too. The Milan team's most radical claim: the referring agency, the school and the clinic hold premises that participate in the problem. A school that defines a child as "disruptive" and a family that defines him as "spirited" are in a conflict of premises, and the conflict is part of the system that maintains the difficulty.

Hypothesising. The therapist and team form provisional explanations — several at once — and hold them lightly. A hypothesis is not a diagnosis; it is a guide to questions, to be abandoned when the answers do not fit. The discipline is to have at least two hypotheses at all times, so that no single one becomes a conviction.

Circularity. The therapist's capacity to think in loops and relationships rather than in causes and essences: to ask about differences, about who does what with whom, and about how each person sees the relationships between the others. Circularity is the model's method and its epistemology: the therapist is part of the system being described, not outside it.

Neutrality. Not indifference, and not the absence of values: a curiosity directed at all the descriptions in the system, including the therapist's own. The Milan sense is "neutral toward the effects of the intervention, and committed to exploring all the premises in the room". The distinction matters ethically (6.7).

Positive connotation. Attributing a benign, system-serving purpose to the family's behaviour — including the symptom — so that the family's resistance is not engaged. "Your daughter's crises may be the way this family stays together" is a positive connotation. Its logic is that a family cannot be blamed for doing something that holds the family together; its hazard is that it can be heard as a licence.

Rituals. Prescribed actions, often involving the whole family, performed between sessions and framed as marking a transition or honouring a rule. Rituals were the early Milan intervention of choice, and they are the ancestor of the modern therapeutic ritual in narrative and systemic practice.

The reflecting team. Tom Andersen's development, adopted by later Milan practitioners: the team talks about the family in the family's presence, offering reflections as possibilities rather than verdicts, while the family listens. The family then chooses what

was useful. The format is the most direct institutionalisation of the model's epistemology — multiple descriptions, no final authority — and it has spread into training, supervision and everyday practice.

The "irreverent" later style. Cecchin's later insistence that the therapist should be irreverent toward all hypotheses, including their own, and that the aim is to keep the conversation going rather than to arrive at a truth. This is the point at which the model becomes closer to social constructionism than to systems theory.

6.2a The family premise, found in three questions

The concept of the premise is the model's most distinctive idea and the hardest to teach. Three questions that surface one in a first session.

Question 1 — the rule question. "What is the one thing in this family that nobody is allowed to say?" The question is disarming, and families usually answer it with a laugh and then with something true. The answer is a premise in the family's own words: "you cannot criticise my mother"; "we do not talk about money"; "nobody in this family is allowed to be weak".

Question 2 — the inheritance question. "What did each of you learn in the house you grew up in about what a family must never do?" The answers are the premises the parents brought, and the differences between them are often the marriage's central difficulty.

Question 3 — the cost question. "What has it cost this family to keep that rule?" The question is the one that makes the premise workable, because a premise that costs nothing can be left alone. The mother who says "it has cost me my daughter" has named the premise's price, and the session has a subject.

A worked example, from the Adeyemis. The rule question produces "nobody is allowed to say that my mother cannot manage". The inheritance question produces, from the mother, "in my house, you did not complain, because my mother had lost her husband and we had no right to add to it"; and from the father, "in my house, you did not stay if there was trouble". The cost question produces, from the mother, "it has cost me my daughter's confidence in me".

What the three questions produce. A premise stated in the family's own words ("nobody is allowed to say that my mother cannot manage"), its transmission ("we had no right to add to it"), and its price ("my daughter's confidence"). That is the material for a positive connotation, a circular inquiry, and a plan.

What the therapist does with it. Does not argue with the premise, does not interpret it, and does not ask the family to abandon it. Instead: "so the rule has been that nobody adds to what she is carrying. And the cost has been that your daughter could not tell you what she was carrying. What would it take to change the rule without dishonouring your mother?" The premise is respected; the rule is opened; the family does the rest.

6.3 The three-part stance

Hypothesising, circularity and neutrality are taught as a single stance with three facets, and each facet has a practice.

Hypothesising, in practice. Arrive with three hypotheses, share some of them with the family as questions, and revise constantly. The discipline is to notice when you have stopped hypothesising and started believing.

Three hypotheses for the Adeyemis, held at once. (1) The school refusal protects the mother from a conversation about her own grief. (2) The refusal is Amara's protest at being the family's emotional manager, and her absence forces the parents to talk to each other. (3) The refusal is an ordinary adolescent difficulty that the school's escalation has inflated into a crisis, and the family's response to the institution is the problem. Each is plausible; each generates different questions; the answers discriminate between them.

Circularity, in practice. Ask about differences, relationships and comparisons rather than about facts. The questions in 6.5 are the technique.

Neutrality, in practice. Be equally curious about every description, including the one you find least sympathetic. When you notice that you have stopped being curious about a particular member, that is the moment to ask them a question.

Why the three belong together. A hypothesis held without circularity becomes a conviction; circularity pursued without neutrality becomes an interrogation of one member; neutrality without a hypothesis becomes a pleasant, aimless conversation. The stance is the combination, and it is what the model's observers look for when they assess a trainee.

6.4 Positive connotation, and its logic

Positive connotation is the model's most distinctive intervention and the one most often performed as a compliment. It is a technical move with a specific structure.

The structure. It attributes a purpose to a behaviour in the service of the system, and it addresses the whole family rather than one member. "The two of you have kept the argument about the school between you, and it has meant that neither of you has had to face the thing you are both most afraid of" — the behaviour (the argument), the purpose (avoiding a topic), and the system (the couple) are all named.

The logic. A family that is blamed for a symptom must defend itself, and the defence consumes the therapy. A family that is told its behaviour serves the family's cohesion cannot defend itself in the same way, because the statement is respectful. The resistance that the strategic school tried to interrupt is here dissolved by a description the family can accept.

The hazard, stated plainly. Positive connotation can be heard as permission: "the therapist says our daughter's crises keep the family together, so we will keep having them." The safeguard is the second half of the intervention — having connoted the behaviour positively, the therapist prescribes something that changes it, or asks what would allow the family to have the cohesion without the symptom. A positive connotation without a second move is an aesthetic gesture.

The ethical limit. Positive connotation is unacceptable where the behaviour is harmful: violence, abuse, an eating disorder in acute phase, self-harm. A therapist who says "your

husband's violence has been the way this family avoids its conflicts" has produced a sentence that is theoretically elegant and professionally indefensible. The model's own literature is thin on this, and MFT 701 Week 12 supplies the rule: the professional obligation overrides the technique.

6.4a A positive connotation, built line by line

The technique is easy to describe and hard to perform. This section builds one slowly, with the sentences a trainee can actually say, and then shows the same sequence performed badly.

The family. The Okonkwo household: Ebuka (9), his mother Ada (34), his father Chuka (39), and a baby of eleven months. Ebuka's defiance has worsened since the baby arrived. He refuses to do homework, answers his mother sharply, and has twice walked out of the house at night. The parents argue about discipline in front of him, and the paternal grandmother, who died in 2023, was the person who used to settle these disputes.

Step one — describe the behaviour precisely. Not "Ebuka is defiant", which is a verdict, but "when his mother asks him to do homework he says no, and when she raises her voice he leaves the room; twice he has gone into the street". The description is behavioural, dated and observable, and it contains no motive.

Step two — state the system, including the problem. "This is a family in which the father works nights, the mother is alone with two children all evening, and the person who used to settle disagreements has died." The statement is factual, and it puts the behaviour in a context without excusing it.

Step three — attribute a purpose that serves the system. "I want to say something that may sound strange. It seems to me that Ebuka's behaviour is doing something for this family. When he refuses, his parents come together — they argue, but they argue with each other about him rather than about the fact that they are both exhausted and that the grandmother is not here to help. In a way that nobody intended, his defiance is the thing that keeps the family talking about the same subject."

Step four — attach the second move. "If that is even partly right, then the question is what the family would need in order to come together without Ebuka having to provide the occasion. And that is a bigger question than homework." The second move converts a compliment into a connotation: it names the price, and it introduces the idea that the behaviour can be retired.

Step five — deliver it tentatively and then be quiet. "I may be wrong about this. I would be interested in what each of you thinks." Then wait. The family's response is the material for the next twenty minutes, and a therapist who fills the silence has wasted the intervention.

The same sequence, performed badly.

Bad step one. "Ebuka is a very difficult child who is testing his mother." This is a verdict with a motive attached, and the family will spend the session defending or attacking it.

Bad step two. "This is a family where the mother is overwhelmed." A description of one member, delivered as a fact, which will be heard as criticism by the mother and as an alliance by the father.

Bad step three. "Ebuka is being naughty because he wants attention." A psychological explanation of an individual, not a systemic connotation: it locates the purpose in the child's wish rather than in the family's organisation.

Bad step four. No second move. The family hears "your son is being difficult for good reasons", which is a justification, and the session ends with the parents feeling that the therapist has taken the child's side.

Bad step five. The therapist explains the connotation at length, defends it against the father's objection, and wins the argument. The family leaves with a formulation they did not ask for.

The four sentences that distinguish the good version. "It seems to me that…" (tentative); "in a way that nobody intended…" (non-blaming); "the question then is…" (second move); "I may be wrong" (revisable). A trainee who memorises these four phrases and uses them in order will perform the technique recognisably, and the nuance can be added later.

6.5 Circular questioning

The model's most transferable skill. Six types, each with a purpose and an example.

Type 1 — the difference question. Asking about differences between people, times or behaviours. "What is the difference between the way your mother treats you and the way she treats your sister?" Purpose: it makes a relationship visible without asking anyone to describe a feeling.

Type 2 — the relationship question, asked of a third party. "If your brother were here, what would he say your father does when you cry?" Purpose: it obtains a description of a relationship the speaker is not in, which is often freer and more informative than a self-report.

Type 3 — the ranking or scaling question. "On a scale of one to ten, how close are your parents? And how close would they say they are?" Purpose: it produces a comparison, and the difference between the two answers is the data.

Type 4 — the before-and-after question. "What was different in the house before your father lost his job?" Purpose: it locates a change in time and invites a description of the pattern rather than of the event.

Type 5 — the hypothetical or future question. "Suppose the school problem were solved, what would be the next thing the family would have to deal with?" Purpose: it tests the function hypothesis, and the family's answer is often the most useful sentence of the session.

Type 6 — the observer question. "Who in the family is most worried about your mother's sadness?" and "Who would notice first if things improved?" Purpose: it maps the family's emotional distribution and its alliances in a single move.

How to tell a circular question from a factual one. A factual question asks what happened; a circular question asks about a difference, a relationship or a comparison. "Does your mother cry?" is factual. "Who notices first when your mother is about to cry?" is circular. "Why does your mother cry?" is neither; it is an invitation to a theory.

The practice standard. The laboratory requires ten minutes of circular questions with 80% genuinely circular. Most trainees achieve 50% on the first attempt, because under pressure they revert to facts — and the observer's count is the most useful feedback in the week.

6.5a Twenty circular questions, grouped

A resource for the laboratory, and a revision list for the examination.

About worry and noticing. Who is most worried about X? Who would notice first if X improved? Who does X confide in, and who does X protect from it? Who in the family would say this problem is not serious?

About differences. What is the difference between how your mother treats you and how she treats your brother? What is different about the days when it is better? What is the difference between what your father says and what he does?

About relationships seen from outside. If your sister were here, what would she say is the trouble? What would your grandmother say about how this family handles money? If I asked your teacher, what would she say has changed in you?

About time. What was different in the house before the move? When did you first notice that your wife had stopped asking? What will be different in a year if nothing changes?

About the future and the hypothetical. Suppose the problem were solved tonight, who would notice first tomorrow? Suppose your son were suddenly fine, what would the family have to deal with next? If you had to choose one person in this room to solve this, who would it be, and why?

About the system's own view. Who in this family carries the worry, and who carries the practical things? Who would be most upset if the problem disappeared? Which of you would find the change hardest?

How to use the list. Not as a script: as a repertoire. Choose the family's least powerful member and ask them two questions from "about relationships seen from outside". Then choose the family's most certain member and ask them "who would be most upset if the problem disappeared?" The two answers will tell you more about the system than an hour of history.

The one question to remember. "Who would be most upset if the problem disappeared?" It is the model's most powerful single question, it tests the function of the

symptom without asserting it, and the family's answer is often a silence followed by a name.

6.6 Annotated transcript: circular questions in a first session

A twelve-turn sequence with the Ogundipe family: mother (44), father (47), daughter (16) — the identified patient, who has been "withdrawn" for six months — and son (12). The therapist asks only circular questions for the first ten minutes.

> **T1 Therapist:** I want to start somewhere unusual. Before I ask anyone what the problem is, I want to ask what each of you would say the others are most worried about. Mrs Ogundipe, what is your husband most worried about?
> **T2 Mother:** (pause) He is worried about her. He thinks she is depressed.
> **T3 Therapist:** And what would he say you are most worried about?
> **T4 Mother:** That she will not talk to me any more.
> **T5 Therapist:** Mr Ogundipe, your wife says you think your daughter is depressed. Who in the family is most worried about her?
> **T6 Father:** Her mother. Definitely.
> **T7 Therapist:** And who would your daughter say is most worried?
> **T8 Father:** (pause) Her mother.
> **T9 Therapist:** (to daughter) Your father has just said something about you as though you were not here. Let me ask you. When your mother is worried about you, what does she do that your father does not do?

> **T10 Daughter:** She comes into my room.
> **T11 Therapist:** And when she comes into your room, who notices first — your father, or your brother?
> **T12 Daughter:** (small smile) My brother. He listens at the door.

Annotations. T1 is the observer question, used as an opening. It does two things at once: it refuses the conventional "tell me about the problem", and it establishes that the therapist is interested in relationships rather than in reports. The mother's answer at T2 is already a description of a relationship.

T3 is a reciprocal question, and it produces an asymmetry that is clinically interesting: the husband is worried about the daughter, the wife is worried about the relationship with the daughter. The difference between the two worries is the material for the session.

T5–T8 is a nested comparison: the father's view of who worries, and his prediction of his daughter's view. The fact that the two are identical is not surprising; the fact that he answers both without hesitation suggests a settled picture of the family that nobody has tested.

T9 is the most important move in the transcript, and it is a correction delivered as an invitation. The father has spoken about his daughter in her presence; the therapist names it lightly ("as though you were not here") and hands the question to the daughter. This is

circularity in the service of inclusion: the model's questions are designed to distribute speech, not only to gather information.

T11 is a relational question asked of the least powerful member about two others, and T12 produces a moment of humour and a piece of structural information — the brother is the household's listener, and he is the family's information channel. Nobody had mentioned him.

What the therapist did not do. Did not ask what the daughter felt; did not ask the parents to explain their worries in more detail; did not offer a formulation. The questions were the intervention, and the family's own account of itself has already changed by the end of ten minutes.

6.7 Neutrality, and its ethical limit

The model's most contested concept, and the one on which the essay will focus.

What neutrality is in Milan. Cecchin's definition: curiosity toward all the descriptions in the system, including the therapist's own, and neutrality toward the effects of one's interventions. It is an epistemic discipline, not a moral position, and it has two purposes: it keeps the therapist's hypotheses provisional, and it prevents the family from organising itself around the therapist's preferences.

What neutrality is not. It is not the absence of values, not the suspension of judgement, and not a refusal to intervene. A therapist who is neutral about whether a child is safe has misunderstood the concept.

Where the model's literature is insufficient. In cases of violence, abuse and coercion, the model's own texts are thin. The theoretical claim — that all descriptions in the system are equally available for curiosity — has been read as a claim that all descriptions are equally valid, which is a different and dangerous proposition. The distinction that resolves it: curiosity is owed to every person; validity is not. A therapist can be curious about a violent man's account of his marriage, and must not treat his account and his wife's account as equally credible descriptions of whether she is safe.

The three-part rule for practice.

Rule 1 — neutrality toward meaning, not toward harm. Be equally curious about all descriptions of the relationships; be unequivocal about the safety of the people in the room.

Rule 2 — say the safety position aloud. Where violence is present, state the position in plain language, once, and then continue the systemic work. Silence about harm in the name of neutrality is the failure mode.

Rule 3 — the therapist's own premise is also subject to curiosity. "I notice I have assumed that a wife should be able to speak freely in her own home. What does this family assume?" The question is not rhetorical; asking it has changed the therapy in more than one published case.

> **CAUTION**
> Circular questions about a violent incident ("what does your wife do that makes you lose your temper?") invite the victim to be described as the cause. Ask them to the person who used violence, about their own behaviour, and never in a form that distributes responsibility to the person who was hurt.

6.7a Neutrality in a crisis: a three-minute script

The moment the model's critics point to, and the one trainees most often get wrong. A workable sequence, rehearsed in the laboratory.

The situation. A couple's argument has escalated; one partner has pushed the other against a wall. Both are in the room. The therapist is working systemically, and the couple wants to talk about the argument's causes.

Minute one — stop the systemic work. "I am going to pause the questions for a moment, because something you have described matters more than the argument." The sentence does not abandon the model; it names a limit the model has, and it says plainly that the therapy has a hierarchy of concerns.

Minute one, continued — state the position once. "What you have described is physical force. In this room I am not neutral about that. Nobody is expected to be safe by being careful, and it is not your job to make sure it does not happen again." The second sentence is addressed to both partners, and it is the sentence that prevents the victim from being recruited into the work of prevention.

Minute two — name the process, not the person. "What I am curious about, and will want to ask about, is what happens in this relationship in the hour before the argument. That is not the same as saying the argument is anyone's fault." Curiosity is reinstated with its limit visibly marked.

Minute three — make the safety plan concrete. Where does each partner go when the argument begins to escalate; who can be telephoned; what happens to the children; and what will the couple do tonight. Then return to the couple's own account, with the plan in place.

What not to do. Do not ask "what did you do that made him angry?"; do not offer a systemic formulation that distributes responsibility before the safety position is stated; do not offer the position as a preference ("I would prefer that we…") — it is a limit, and it is stated without apology.

The follow-up. Record the position in the notes, with the date and words used, and revisit it at the next session. A stated position that is never mentioned again is a rhetorical gesture; a stated position that is revisited is a structure.

6.8 The reflecting team, and the modern non-directive version

The Milan format's most important descendant, and the version most trainees will actually use.

The original format. A team observes behind a one-way screen; the therapist interviews; the team telephones or the session breaks; the team's intervention (a positive connotation and a prescription) is delivered by the therapist. The family receives one authoritative description.

The reflecting team. Tom Andersen's revision: the team talks in front of the family, offering several descriptions as possibilities, and the family listens and then chooses what was useful. The authority moves from the team to the family, which is a direct expression of the model's epistemology.

How to run one in a training clinic, in six steps.

1. Prepare the family. Explain the format, obtain consent, and say what will happen and what will not be recorded.
2. Interview for twenty to thirty minutes, while the team listens without speaking.
3. Swap. The therapist joins the team; the family listens to the team's reflections.
4. Reflect in hypotheses, not verdicts. Each team member offers a description, in the first person, briefly, and without arguing with one another.
5. Swap back. The therapist asks the family which reflections fitted, which did not, and what they would add.
6. Close with the family's choice. What will they take from the reflections, and what will they do differently?

The rules of good reflection. Speak about the family with respect, in the family's own language where possible; offer several descriptions rather than one; avoid advice; keep each contribution short; and do not use the format to deliver a verdict that the interviewer did not want to give.

Why the format is valuable beyond the model. It makes the therapist's thinking visible and revisable; it distributes authority; it models uncertainty; and it gives the family an experience of being thought about carefully by several people, which many families describe as the most useful part of the therapy.

The cost. It requires two or three clinicians, which most services cannot spare. The practical adaptation for a single therapist is a reflecting conversation with a colleague by telephone or in a later session, or a written reflection read to the family — both are legitimate descendants of the format and both are used in training clinics.

6.8a A reflecting team, run with three trainees

A fishbowl adaptation for a course without a one-way screen, and the debrief that makes it teachable.

The setup. Four trainees and a tutor. One trainee interviews a volunteer family (recruited from the course, briefed with a scenario); the other three observe from the corner of the room, in the family's sight. After twenty-five minutes the interviewer joins the observers, and the four talk for seven minutes while the family listens. The interviewer then returns to the family for five minutes to ask what fitted.

The observers' brief. Each offers one description, in the first person, of no more than forty seconds, using the family's own words where possible. No advice, no diagnosis, no agreement with one another, and no argument. The tutor keeps time and stops any contribution that becomes a verdict.

What actually happens, in the first attempt. The observers almost always begin with interpretations of the family ("I thought the mother was depressed"), then, corrected, move to descriptions of relationships ("I noticed the father answered for his wife three times"), and finally, in the last two minutes, produce the kind of reflection the format is for ("I wondered whether the father answers for his wife because he is protecting her from something, and I noticed that the wife did not correct him"). The progression is the pedagogy: the trainees learn that the useful reflections are relational and tentative, and they learn it by hearing themselves.

The family's part. The volunteer family is asked three questions afterwards: which reflection fitted, which did not, and what they would add. In the course's first run, the family chose the reflection the observers thought was weakest, and could say exactly why — which is the lesson about authority that the format is designed to teach.

The debrief, in the order that works. First the family speaks, then the observers, then the interviewer. The interviewer's question is always the same: what did you hear that you could not have said yourself? The answer is usually a description that the interviewer had formed and suppressed, and the discovery that the team can say it is the beginning of an understanding of what the format is for.

The written variant, for a single therapist. Write three reflections after the session, in the first person, and read them aloud at the start of the next: "I have been thinking about our conversation, and three things occurred to me…" The family chooses. It is a thin version of the format and it works; several published single-therapist adaptations describe exactly this.

6.9 Laboratory: ten minutes of circular questions

The week's practical, sixty minutes, in triads, with a strict rule that makes the skill visible.

Setup. Two peers play a family (a parent and an adolescent), briefed with a simple scenario and one premise each — for example, the parent believes that a good child does not answer back; the adolescent believes that the parent's silence means disapproval. The therapist may ask only circular questions for ten minutes. The observer counts.

The count. The observer classifies every question as circular (about difference, relationship or comparison), factual (about what happened or what someone is like), or leading (containing the answer). The target is 80% circular. The count is read out at the end without comment, and the therapist's reaction to the count is often the most instructive part of the exercise.

The debrief questions. Where did you revert to facts, and what was happening in the room at that moment? Which circular question produced the most information? Which one did the family find uncomfortable, and why?

Marking rubric.

Criterion	Weight	Full marks
Circularity	35%	At least 80% of questions genuinely circular; all six types used at least once
Use of difference	20%	Questions about differences between people, times and relationships, not about essences
Distribution	20%	Speech distributed; the least powerful member asked as often as the most
Neutrality	15%	Curiosity visible toward every member's description, including the least sympathetic
No interpretation	10%	The therapist offers no formulation, no advice and no reassurance

Why the last criterion matters. Trainees who are anxious to be helpful begin to interpret in minute four, and the interpretation converts a circular interview into a conventional one. The prohibition is the pedagogy: it forces the therapist to stay in the question, which is where the model's work is done.

6.10 Six errors in Milan work

1. **The circular question used as an interrogation.** A sequence of clever questions with no warmth, which the family experiences as an examination. Fix: questions in the service of curiosity, with the family's answers acknowledged.
2. **The abandoned hypothesis.** Arriving with one hypothesis and asking only the questions that confirm it. Fix: three hypotheses at all times, and a deliberate question for each.
3. **The compliment mistaken for a connotation.** "You are a very caring family" is not a positive connotation. Fix: the structure of behaviour, purpose and system, with a second move attached.
4. **Neutrality as silence about harm.** Refusing to name a safety concern because naming it would be non-neutral. Fix: the three-part rule in 6.7.
5. **The reflecting team as a verdict.** A team that agrees on one description and delivers it. Fix: several descriptions, in the first person, offered as possibilities.
6. **The expensive format.** Refusing to work without a team, and therefore not working. Fix: the single-therapist adaptations, used deliberately and recorded.

6.10a Three circular interviews that failed

A failure typology, drawn from recordings in a training clinic, because the model's failures are more instructive than its successes.

Failure one — the beautiful interview. A trainee produced forty flawless circular questions in thirty-five minutes. The family answered every one. Nothing changed, and the family did not return. The debrief found the cause: every question was asked of the parents, and the fourteen-year-old, who had said nothing at all, had been described throughout. The interview was elegant and the system had not been entered, because the

person with the least power had not been given a voice. The rule broken: distribution. Circularity without distribution is a questionnaire.

Failure two — the hypothesis with a destination. A trainee arrived with a hypothesis about a family's enmeshment and asked fifteen questions that each confirmed it. The family, obliging, produced evidence. At minute twenty the trainee delivered a positive connotation of the mother's over-involvement which the mother heard as a diagnosis of her parenting. The father's single remark — "she has been like this since her own mother died" — was the material for a second hypothesis that was never formed, because the trainee had one. The rule broken: hypothesising. One hypothesis is not a hypothesis; it is a conclusion with questions attached.

Failure three — the neutrality that was silence. A family presented with a father who had broken a door in an argument. The trainee, determined to be neutral, asked circular questions about everyone's view of the door, and did not say that a broken door in a household with two children is a safety matter. The mother answered carefully and did not come back; the father later attended alone and said the therapist "seemed to think it was normal". The rule broken: the ethical limit. Curiosity about all descriptions, offered without a stated position on harm, is heard as collusion.

The pattern across the three. Each failure is a virtue taken too far: circularity without distribution; hypothesising without plurality; neutrality without a position. That sentence is the week's summary, and it is worth writing at the top of the page in the examination.

How to catch each one in the room. For distribution: count who has been asked, and ask the person who has been asked least — usually the child, or the father who has said nothing. For plurality: ask a question whose answer would falsify the hypothesis, aloud, as a genuine enquiry. For neutrality: state the safety position once, in plain language, at the point where the material requires it, and then continue.

6.11 Culture, faith and the Milan stance

The model's cultural profile is subtle: its epistemology travels well, and its historical practice less so.

Where it travels well. Curiosity toward all descriptions is a resource in cross-cultural work, because it prevents the therapist from treating their own cultural account as the standard. The circular question is also unusually effective across difference: it asks about relationships, which every family has, rather than about feelings, which are described differently in different cultures. A question such as "who would notice first if things improved?" works in any family, in any language, with any premise.

Where it strains. Three features.

The team format assumes a particular kind of clinic. A one-way screen, a team of three and a family willing to be observed are the conditions of a European training institute. In a community clinic, a rural hospital or a family's home, the format is unavailable, and a therapist who insists on it will not work.

The premise that all descriptions are equally available for curiosity can look like a refusal to take a position in families where a religious authority has already pronounced. The therapist's curiosity is not a challenge to that authority, and it must not be presented as one.

The abstention from advice can be experienced as unhelpfulness in families who expect direction, particularly where the therapist is younger than the parents. The model's answer is that the questions are the help; the honest answer is that some families need something else first, and a therapist who cannot give it will lose them.

Faith and the reflecting team. The format is unusually hospitable to a family's spiritual account, because it allows several descriptions to coexist. A team that includes a family's spiritual explanation among its reflections — "one thing I wondered about was what the family's faith says about this" — is doing the model properly, and is more likely to be heard than a therapist who either adopts or contests the account.

> **DEBATE**
> "Milan neutrality is a luxury of well-resourced clinics and an evasion in under-resourced ones." Argue both sides with reference to a case.

6.11a Hypothesising about a family's faith, without appropriating it

The Milan stance is often taught as though religion were one more premise to be circulated. This section offers a discipline for doing it well.

The distinction. A family's faith can be a premise ("we do not let the sun go down on our anger"), a resource (the church's marriage group, the imam's counsel), an authority (a teaching that settles a question the family cannot settle), or a site of conflict (one spouse's practice has changed). The therapist's questions, and their ethics, differ for each.

Hypothesising about a premise. "In this family, what does your faith say must never happen between a husband and a wife?" The answer names the premise in the family's own vocabulary, and it can then be circulated: "who in the family holds that most firmly? who would say it is the thing that has held the marriage together?" No interpretation is offered, and the premise is not relativised.

Hypothesising about a resource. "Who in your congregation knows what is happening at home, and what have they said?" The question often reveals an untapped resource and occasionally a second problem: a family that has told nobody, because the appearance of the marriage is part of its religious obligation.

Hypothesising about an authority. "If your pastor were sitting here, what would he say about this?" The question is circular, hypothetical and respectful, and it produces the family's own account of the authority's position — which is often more nuanced than the therapist assumed, and sometimes more severe.

Hypothesising about a conflict. "When your practice changed, what did your husband do that told you he had noticed?" The question locates the change in the relationship rather than in doctrine, and it keeps the therapist out of the theological argument.

The three prohibitions. Do not adjudicate a theological question; do not use the family's faith as evidence for a psychological formulation ("her guilt is really religious scrupulosity"); and do not offer a spiritual interpretation of your own unless the family has invited it.

The one invitation that is usually welcome. "If it would be useful, I can ask whether your church's counselling team has worked with families in this situation." The offer is practical, it respects the family's own structures, and it makes the therapist's position clear: an ally of the family's resources, not a rival to its faith.

6.12 Evidence note: a stance rather than a treatment

The matrix cell for this model requires care, because the model's influence is greater than its trial evidence and its components are more evidenced than its package.

What the evidence is. There are very few randomised trials of Milan systemic therapy as a package. The early Milan work is reported as case series, often with dramatic outcomes and no comparison group. The model's later developments are documented in theoretical and clinical literature rather than in trials.

What the components have. Circular questioning and systemic hypothesising are widely used and studied qualitatively; reflecting teams have a modest research literature, mostly qualitative and process-focused, with some evidence that families value the format; systemic practice more broadly has trial evidence in several presentations (MFT 701 Week 11). The concept of the family premise is a clinical heuristic rather than an established mechanism.

What the matrix cell should say. "No adequate trial evidence for the package; strong influence on the field through the circular question and the reflecting team, both of which have qualitative support; the stance is a component of practice across models rather than a treatment in its own right."

The practitioner's claim. "I use the stance — hypothesising, circularity, neutrality — because it improves the quality of my questions and reduces my certainty, and I use the reflecting team where a team exists, because families report it as useful. I do not claim that Milan systemic therapy is an evidence-based treatment for a specific presentation, and I measure my own outcomes." That sentence is honest, defensible and — in the viva — distinctive, because most candidates overstate.

6.13 Key terms

Term	Definition
Premise	An unexamined family assumption that organises behaviour
Hypothesising	Holding several provisional explanations at once and revising them
Circularity	Thinking and asking in loops, differences and relationships rather than causes and essences
Neutrality (Milan)	Curiosity toward all descriptions in the system, including the therapist's

Term	Definition
Positive connotation	Attributing a benign, system-serving purpose to a behaviour, with a second move attached
Circular question	A question about difference, relationship or comparison
Observer question	A question asking who notices, who worries, who would see a change first
Ritual	A prescribed action performed between sessions, marking a rule or a transition
Reflecting team	A team that reflects in the family's presence, offering possibilities rather than verdicts
Irreverence	Cecchin's later stance: holding no hypothesis, including one's own, as final
Second-order cybernetics	The observer is part of the observed system (MFT 701 Week 1)
Team break	The interval in which the team forms its intervention

6.14 Self-test

1. What are the three parts of the Milan stance, and what does each protect against?
2. What is a premise, and give one example from a family you know?
3. Distinguish a circular question from a factual one, with an example of each.
4. Name the six types of circular question.
5. What is the structure of a positive connotation, and what must accompany it?
6. What is the ethical limit of neutrality, and how is it stated in practice?
7. How does a reflecting team differ from the original Milan format?
8. What are the three parts of an honest evidence claim for this model?

Answers. 1. Hypothesising (protects against conviction), circularity (protects against linear and essentialist thinking), neutrality (protects against the therapist's preferences organising the family). 2. An unexamined assumption that organises behaviour, e.g. "conflict destroys love", which produces a household where nothing can be discussed. 3. "Does your mother cry?" is factual; "who notices first when your mother is about to cry?" is circular. 4. Difference; relationship asked of a third party; ranking or scaling; before-and-after; hypothetical or future; observer. 5. It attributes a purpose to a behaviour in the service of the system; it must be accompanied by a second move that changes something, or it is a compliment. 6. Curiosity is owed to every person but validity is not equally owed to every account; where harm is present the safety position is stated aloud once, plainly, and the systemic work continues. 7. The original delivered one authoritative team intervention; the reflecting team offers several descriptions in the family's presence and lets the family choose. 8. No adequate trial evidence for the package; qualitative and process support for the circular question and the reflecting team; the stance is a component of practice across models.

6.15 Reflective task

Take a disagreement in your own family and write down every member's description of it, in their own words, without evaluating any of them. Then write one paragraph on which description you found hardest to be curious about, and why. Bring both to the seminar. The exercise is the model's stance applied to the therapist, and most students discover that their curiosity has a boundary which coincides with their own history.

6.16 Readings for this week

- **Palazzoli, Boscolo, Cecchin & Prata (1978)**, Paradox and Counterparadox, the original statement, read for the team format and the rituals.
- **Palazzoli, Cirillo, Selvini & Sorrentino (1989)**, Family Games, for the later elaboration.
- **Cecchin (1987)**, "Hypothesizing, circularity, neutrality revisited", Family Process — the single most important paper for this week.
- **Tomm (1987a, 1987b)**, "Interventive interviewing", parts I and II, Family Process, for the classification of circular questions.
- **Andersen (1987)**, "The reflecting team: dialogue and meta-dialogue in clinical work", Family Process.
- **A recorded reflecting team session**, watched for how the team speaks about a family in the family's presence.

Read the Cecchin paper first and twice: the second reading is the one in which the three concepts stop being a checklist and become a stance. Read Tomm with a transcript beside you and mark every question by type — the exercise takes an hour and changes how you listen. Read Andersen for the format's ethics rather than its mechanics. The Palazzoli texts are best read as history: they show a brilliant clinical team inventing a method, including its mistakes, and the later chapters show the team revising itself. For the essay, cite Cecchin and Tomm for the stance and Andersen for the format, and be candid that the package has no trial evidence and that its components are supported qualitatively. A reading log with three sentences per source — what it argues, what it assumes, what it cannot show — is the most useful artefact you can bring to the seminar, and it will serve you again in the viva, where examiners ask less about what Milan therapy is than about what you do when its stance meets a family in danger, a family of another faith, or a clinic with no team and no screen.

Next: Week 7 — Experiential and human validation models

Week 7 — Experiential and human validation models

The question. What would happen if this family could feel and say the truth to each other, right now?

Key figures. Virginia Satir (family sculpture, communication stances, self-worth, "the five freedoms"); Carl Whitaker (symbolic-experiential therapy, absurdity, the therapist's use of self); Kempler and the Gestalt-influenced tradition.

Core concepts. Growth rather than pathology; the present moment as the site of change; expression of authentic feeling; the therapist's own aliveness as a therapeutic instrument; family sculpture as non-verbal enactment of structure.

Signature techniques.

- **Family sculpture:** family physically arrange themselves to show the emotional map; then rearrange.
- **Communication stances** (Satir): placating, blaming, super-reasonable, irrelevant, and congruent.
- **Here-and-now enactment:** "Tell her now. I want to see what happens to your face when you say it."

Contribution. Humanised the field; insisted on warmth, humour and the person of the therapist. Many of its insights survive inside EFT and other emotion-focused models.

Blind spots. Dependent on the charisma and health of the therapist; thin manualisation; risk of emotional flooding without containment.

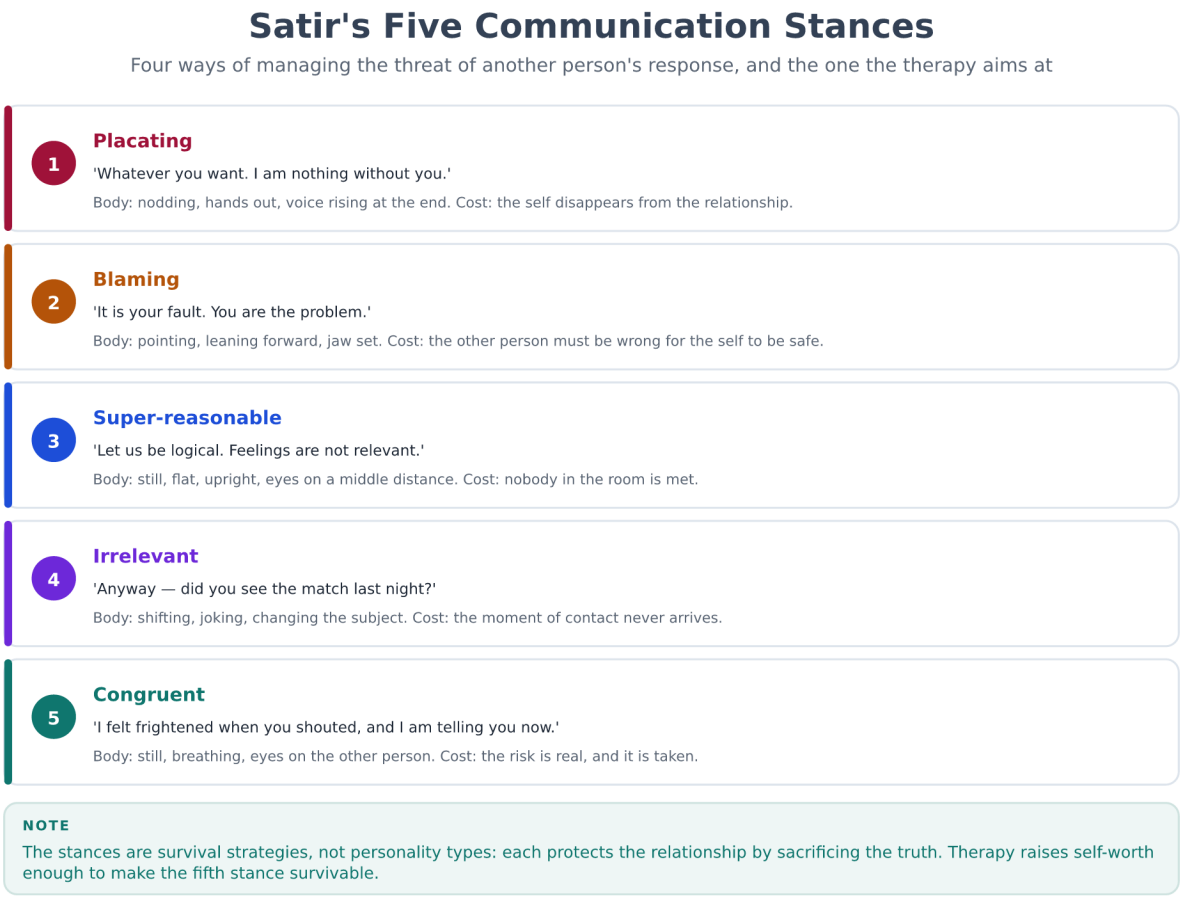


Figure 7.1 — Satir's five stances: four ways of managing the threat of another person's response, and the one the therapy aims at

7.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	From Milan to the living room	Where the model came from, and why it distrusted technique	Name the difference from Week 6
10–25	Satir: self-worth and the stances	The five stances, demonstrated with three chairs	Identify each stance in a recorded clip
25–40	Whitaker: the therapist as instrument	Absurdity, the use of self, the risk of charisma	Write one sentence on their own therapeutic presence
40–55	Sculpture	A volunteer family, sculpted twice	Sketch the sculpture and what changed
55–70	Here-and-now enactment	The grammar of the invitation, four moves	Practise the invitation in pairs
70–80	Flooding and containment	When feeling overwhelms the room	Design a containment move
80–90	Critique and matrix row	The model's evidential and ethical position	Fill the row

The teaching idea that carries the week. This is the first model in the course that cannot be taught by explaining it. Every other model so far could be conveyed in a lecture and practised afterwards; this one must be seen — in a recording, in a live demonstration, or in the lecturer's own way of being in the room. A lecturer who explains experiential therapy in a flat voice has taught the opposite of the model.

What the lecture must therefore do. Demonstrate. Sit in a chair and show a blaming stance, then a placating one, then a congruent one, and let the class feel the difference in their bodies before any theory is offered. The theory lands afterwards, when the experience has given it something to attach to.

7.1a Whitaker's absurdity, demonstrated three ways

The model's most misunderstood technique, because it sounds like licence to be strange. Three demonstrations that a lecturer can perform in five minutes each, with the principle behind each one.

Demonstration one — the exaggerated agreement. A student complains that their flatmate never washes up. The lecturer replies, with genuine interest: "you are right, and I think it is worse than you say. I think the flatmate is running a campaign, and the campaign is working." The student laughs, and then says what the flatmate might actually be doing. The principle: absurdity disrupts the problem-saturated account without contradicting it, and the disruption produces new information.

Demonstration two — the deliberate wrong question. To the same complaint, the lecturer asks: "when the flatmate does wash up, what do you do that rewards it?" The question is logically odd — nobody is trained by a flatmate's dishwashing — and it makes the student think about their own part in the pattern. The principle: a question that is slightly off-centre is harder to answer with a rehearsed complaint.

Demonstration three — the therapist's own absurdity. The lecturer says, of their own reaction: "I notice I am oddly invested in this. I think I want the flatmate to be the villain."

The room relaxes. The principle: the therapist's willingness to be a person, including a faintly ridiculous one, lowers the family's need to defend themselves.

The rule that separates absurdity from harm. Whitaker's play is always warm, always in the service of contact, and never aimed at a person's dignity. An absurdity that makes the family laugh with each other is the model; an absurdity that makes them laugh at someone is bullying with a theoretical justification.

7.2 Core concepts

Growth rather than pathology. The model's premise is that families are not broken but blocked: the capacity for growth, intimacy and honesty is present and has been suppressed by fear, habit and the requirements of survival. The therapist's task is not to repair a defect but to create the conditions in which the family's own capacity can appear. This is a strong claim, and it has a clinical consequence: the therapist asks less "what is wrong here?" and more "what is being prevented from happening, and by what?"

The present moment as the site of change. Every model in this course works with the present to some degree; this one makes the present the only site of change. The past explains; the present alters. When a wife describes last Tuesday's argument, the experiential therapist is interested in what happens in her body as she describes it, and in what happens between her and her husband while she is describing it. The question "what is happening right now?" is not a technique among others; it is the model's epistemology.

Self-worth as the organising variable. Satir's central construct. A person with low self-worth cannot say what is true without fearing the loss of the relationship, and so develops a stance that protects the relationship at the expense of the truth. The stances are therefore not personality types but survival strategies, and the therapy aims at a self-worth sturdy enough to bear congruence.

The five stances. Placating: "whatever you want, I am nothing without you." Blaming: "it is your fault, you are the problem." Super-reasonable: "let us be logical, feelings are irrelevant." Irrelevant: the deflection into humour, distraction or another topic. Congruent: the person's words, feelings and body agree, and the person speaks for themselves rather than about someone else. The four incongruent stances share one function: they avoid the risk of a direct statement.

The therapist's aliveness as an instrument. Whitaker's contribution and the model's most contested feature. The therapist's own spontaneity, boredom, warmth, irritation and playfulness are not noise to be filtered out but information and intervention. A therapist who is present as a person offers the family an experience that no technique can produce: an adult who is neither pretending nor withdrawing. The hazard follows immediately: the model's effectiveness depends on the health of the therapist's own person, and a charismatic therapist can produce impressive sessions and dependent families.

Symbolic-experiential therapy. Whitaker's term for a therapy that works at the level of symbol and experience rather than instruction. The therapist joins the family's absurdity, names the unspeakable lightly, and uses the session itself as the event. "The therapy is not a conversation about the family's life; the therapy is a piece of the family's life."

Human validation. The model's ethical core: the family's experience is accepted as valid before it is changed. This is not agreement with every account; it is the refusal to begin by correcting. The stance is closer to Milan neutrality than it first appears, with the difference that the experiential therapist shows their own reaction rather than concealing it behind curiosity.

Enactment. Bringing the relationship into the room and letting it happen in front of the therapist: "tell him now", "show me with your hands", "say that again looking at her". Enactment is the model's bridge between feeling and structure — a concept the structural school (Week 4) also uses, with a different purpose: there, to change boundaries; here, to make contact.

Family sculpture. Satir's signature: the family arranges itself physically into a tableau that represents the emotional map, and then rearranges it. The sculpture externalises what cannot be said, it makes the family's structure visible to the family, and — crucially — the body remembers what the mind edits.

The five freedoms. Satir's statement of the model's values: the freedom to see and hear what is here rather than what should be; to say what one feels and thinks rather than what one should; to feel what one feels rather than what one should; to ask for what one wants rather than waiting for permission; and to risk in the direction of one's own growth rather than choosing safety.

7.2a Two Satir maps, learned by drawing them

The model's concepts are easier to hold if they are drawn. Two maps, both of which a trainee can reproduce from memory, and both of which are useful in a live session.

Map one — the self-worth iceberg. Draw a small triangle above a waterline: at the tip, "behaviour" (what the family sees); below the line, in descending order, "coping stance", "feelings", "feelings about feelings", "perceptions and expectations", and at the base "self-worth". The map makes a single point visually: every argument the family brings is the visible tip of a structure of which self-worth is the floor. The clinical use is a question: "when your wife criticises you, what happens below your behaviour?" Most clients can answer it, and the answer is a feeling about a feeling.

Map two — the temperature reading. Satir's structured check-in, drawn as five small boxes: appreciations, new information, puzzles, complaints with recommendations, and hopes and wishes. The drawing is a session plan; the sequence is deliberate, moving from the easiest material (appreciation) to the riskiest (wishes), so that the family's capacity to speak is built before it is tested. A trainee who runs a temperature reading in a family session has performed the model's core move in twelve minutes, and can then reflect on what happened when the sequence reached "complaints".

Why drawing matters for this model in particular. Experiential therapy is often taught as a set of feelings, which makes it hard to revise. Both maps convert the feelings into a structure with parts, and a structure can be taught, practised, marked and criticised. Draw them by hand during the lecture, not from a slide: the awkwardness of the drawing is itself a demonstration of the model's attitude to technique.

The exercise. In pairs, draw the iceberg from memory in ninety seconds, then explain it to the partner without using the words "self-esteem", "confidence" or "trauma". The prohibition forces the explanation back to the model's own vocabulary — behaviour, stance, feeling, expectation — and the vocabulary is what the examination will test.

7.3 The therapy: warmth, pace and the person of the therapist

Experiential therapy is the least procedural model in the course. Its method is a set of attitudes enacted in the room, and its most important variable is the therapist's own state.

The stance, in five commitments.

1. Contact before content. The session begins with each person being seen: "before we start, I want to know how each of you is, today, in one sentence." The greeting is not a formality; it establishes the model's premise that people are met before problems are solved.
2. The present tense. Descriptions of the past are received, and then the work moves to now: "what happens in your body as you say that?" "Say it to him rather than about him."
3. The therapist's own response, offered sparingly. "I noticed I felt heavy as you were speaking." The disclosure is an instrument, not a confession: it must be brief, about the relation, and in the service of the family's work.
4. Pace. Whitaker's most-quoted contribution: the therapist slows down, waits, and does not fill the silence. The case in the week's stub — the father who has not cried in twenty years — is resolved by pace, not by technique.
5. Play and absurdity. Humour is permitted, and often necessary. A family that can laugh together in the session has already had an experience of connection that the therapy can build on.

What the therapist is doing technically, while appearing not to work. Tracking the level of arousal in the room; noticing which member's body has changed; choosing the moment to invite a statement; holding the frame so that the feeling is contained; and ending each session with the family settled rather than raw. The model looks spontaneous and is in fact highly disciplined.

The two hands metaphor. A useful image for trainees: one hand holds the feeling (warmth, contact, permission), the other holds the frame (time, safety, containment, ending). Therapists trained in technique-heavy models tend to use only the second hand; therapists drawn to this model tend to use only the first. Competence is the simultaneous use of both.

The dependency hazard, stated plainly. Because the instrument is the therapist's person, the therapy can become a relationship the family cannot leave, and the therapist can begin to need the family's response. Signs: sessions that run long by twenty minutes; the therapist's disappointment when a family cancels; the sense that the therapy is the therapist's growth project. The safeguards are supervision that examines the therapist's own material, a caseload that includes non-experiential work, and a written formulation that can be examined by someone else.

7.3a Where the therapist's own history enters the room

The model requires the therapist to be a person. That requirement has a training consequence that the literature discusses less often than it should: the practitioner's own emotional history will shape what they notice, invite and avoid.

The three patterns most often seen in training.

The rescuer. A therapist whose own family required them to manage a parent's distress will move quickly toward comforting, will interrupt expressions of anger, and will produce sessions that feel warm and change nothing. The family leaves soothed and returns with the same problem.

The distancer. A therapist who grew up in a family where feeling was dangerous will keep the session intellectual, will ask about patterns instead of inviting contact, and will describe their work as "systemic" in a way that conceals avoidance. Milan training (Week 6) is a comfortable home for this pattern.

The performer. A therapist who was rewarded for being interesting will produce sessions that are memorable for the therapist's interventions and not for the family's change. The families most at risk are those with least power to resist, and the therapist's caseload will show families who return, admire the therapist, and do not improve.

The discipline that catches each one. Record sessions and review them with a supervisor who is asked a specific question: "what did I avoid in that session?"; keep an outcome measure so that warmth is not mistaken for effectiveness; and, once a term, ask two families directly what they experienced as unhelpful.

A written exercise for the trainee. Write half a page on the feeling you find hardest to sit with in a session — anger, tears, silence, neediness, sexual material, boredom — and one sentence on where you learned to find it hard. Bring it to supervision. This is not therapy, and it should not become therapy; it is the minimum professional maintenance that a model with this premise requires of its practitioners.

7.4 Techniques, with the grammar of each

Family sculpture. Ask the family to arrange themselves physically to represent how things are: who stands near whom, who turns away, who is lifted up, who is left at the edge. Then ask each member, in turn, how it feels to be in that position. Then ask the sculptor to change one thing — usually one thing only — and let the family stand in the new arrangement long enough to feel it. Finally, ask what the new arrangement would require of each person in ordinary life.

The grammar: sculpt; feel; alter one element; feel again; translate into a sentence about daily life. The technique fails when the therapist sculpts for the family, or moves too fast through the last step.

Communication stances. Name the stance being used, ask the person to try its alternative, and let the room feel the difference. "When your wife says 'whatever you want', what happens to you?" — asked of the husband — makes the placating visible. Then: "can you say what you actually want, in one sentence, looking at her?" The

invitation is always small and behavioural. A trainee can rehearse the whole technique in one sentence: name the stance without blaming it, ask the other person what it does to them, and then offer one small alternative and wait.

Here-and-now enactment. Four moves: notice ("I see your jaw tighten when she says that"); invite ("can you say that to her, rather than about her?"); hold ("stay with it a moment"); harvest ("what happened in the room just then?"). The harvest is the part trainees skip, and it is the part that turns an emotional moment into learning.

The temperature reading. The structured check-in described in 7.2a: appreciations, new information, puzzles, complaints with recommendations, hopes and wishes. Useful at the start of a session, at the start of a group, and as a whole session in a family that has stopped speaking.

Parts party. A Satir technique for internal conflict: the person names the parts of themselves in tension ("the part that wants to leave", "the part that cannot"), gives each a voice in the room, and lets the family hear the internal argument. Used carefully, it converts a person's self-attack into something the family can witness with compassion.

Family reconstruction. Satir's longer method for working with a parent's family of origin, using guided enactment of three generations. It belongs to intensive workshops rather than weekly clinic work and is included here because it explains the model's theory of transmission: the parent's unmet needs are the material the child is asked to manage.

The aesthetic use of the room. Chairs, distance, height, standing and sitting — the model treats the physical arrangement as data and as intervention. A therapist who moves a chair has made an intervention; the model knows this and uses it deliberately.

7.4a A sculpture, drawn and then read

Sculpture is the technique trainees find hardest to describe in writing, so this section gives a worked example with a diagram in words and a reading of it.

The family. The Ogundipe family (mother 44, father 47, daughter 16 withdrawn for six months, son 12).

The first sculpture, as the daughter arranges it. The daughter places herself in a corner, facing the wall. She places her mother two steps behind her, facing her back. She places her father at the far side of the room, near the door, facing the room. She places her brother beside the father. The therapist asks each person to hold the position for thirty seconds without speaking.

The reading, in the family's own words. Mother: "I am looking at her back and I cannot reach her." Father: "I am by the door because somebody has to be able to leave." Son: "I am with my father because it is quieter there." Daughter: "I am against the wall because then nobody is behind me."

What the sculpture made visible that three sessions of talk had not. The father's position by the door is not indifference, which is how the mother had described it, but an exit strategy that he had never named; the mother's position is pursuit, which she had

named as care; and the daughter's wall is a defence against being approached from behind, which relates to an incident the family had not mentioned.

The alteration. The therapist asks the daughter to change one thing. She moves her mother from behind her to beside her, still facing the wall, and says "this is better, but I still want the wall." The therapist asks her to stay there for a minute.

The second reading. Mother: "I can breathe." Daughter: "It is not so tight." Father, unprompted: "I would come closer if I thought there was room." The therapist writes that sentence down and reads it back at the next session as the family's own formulation of the change.

The translation step. "What would have to happen at home for your mother to be beside you rather than behind you?" The daughter's answer — "she would have to knock" — is the therapy's first behavioural task and it came from the family, not the therapist.

The drawing task for the seminar. Draw the two sculptures as simple floor plans on one page: circles for people, arrows for facing, distances to scale. Bring the drawing, not a description. The exercise teaches the technique's key skill, which is spatial observation, and it produces a record that a supervisor can read in ten seconds. Draw the second sculpture on a tracing sheet over the first, so that the change appears as a movement on the page rather than as a claim in a report: two circles that were apart are now adjacent, one arrow has reversed, and one person has been given a chair. That single sheet will do more for a case discussion than three pages of prose, and it is the record the family will ask to keep.

7.5 The first session: contact, safety and the first invitation

An experiential first session has a different shape from the structured first sessions of Weeks 4–6.

Phase one, the meeting (ten minutes). Everyone is greeted by name and asked one question that is not about the problem: what they did today, how school was, what they would be doing if they were not here. The purpose is twofold: the therapist learns how each person occupies a room, and the family learns that this therapy begins with people rather than with the problem.

Phase two, the account (ten minutes). Each member is asked, in turn, what they would want the therapist to know, and each is asked to say it without being interrupted. The therapist tracks the body: who leans forward, who looks at whom, who speaks for whom. The tracking notes are the material for the invitation that follows.

Phase three, the first invitation (fifteen minutes). One observation, offered tentatively, becomes an invitation to make contact. "I noticed that when your wife described the last argument, you looked at the floor and your jaw moved. I wondered whether there was something you wanted to say to her that did not get said." If the invitation is declined, it is accepted gracefully, and the session continues. The model's principle: an invitation is offered, never extracted.

Phase four, the harvest (ten minutes). "What happened in the room in the last ten minutes?" Each member answers. The therapist does not interpret the answers; the family's own description of the moment is the session's most valuable product.

Phase five, the frame (ten minutes). What the therapy will be, how often, and what each person can expect; one small piece of contact to practise at home, chosen by the family rather than prescribed; and a clear ending. The model's homework tends to be relational rather than behavioural: "one conversation this week in which nobody is interrupted for three minutes", rather than a chart.

What the therapist does not do in session one. Does not take a full history; does not construct a genogram; does not explain the model; does not attempt a sculpture in the first meeting unless the family invites it; and does not press for tears. The first session's task is to establish that feeling can be spoken in this room without anyone being harmed by it.

The diagnostic question the therapist is asking silently. Can this family tolerate contact? Some families cannot, yet, and a therapist who begins with enactment in such a family will lose them. The test is small: offer one sentence of feeling and watch the response. If the response is deflection, the therapy begins with the deflection — which is itself the material.

7.5a A first-session opening, scripted in eleven sentences

What the model sounds like before any technique appears. Read aloud in pairs; the sentences are short because the model's openings are.

"Thank you for coming, all of you. Before we talk about why you are here, I would like to know each of you a little. Tunde, what did your day look like before you came? Funke? Amara? Segun? Good. Now I would like each of you to tell me one thing you would want me to know, and I would like the others to listen without correcting. Amara, would you start? Thank you. What I noticed while you were speaking was that your mother looked at you the whole time and your father looked at the floor. I am not going to make anything of that yet. I want to ask one question: what would each of you want to be different in this house in six months? Good. Then let us begin there, and I will go slowly."

7.6 Case: the Adeyemis and the unsaid sentence

The week's case, developed across four sessions, with the model's decisions marked.

The presentation. Amara Adeyemi (14) has refused school for five weeks. Her mother, Funke (41), has taken leave from work. Her father, Tunde (46), is in Lagos on a contract and attends by telephone. Her brother, Segun (11), is present and silent.

Session one. The therapist greets everyone, including Segun, and asks each person what they did this morning. Segun says "nothing"; his mother answers for him. The therapist notes it and does not comment. In the account phase, Funke describes the school refusal in detail; Tunde, on speaker, says the problem is "laziness" and that his wife is "too soft". Amara says nothing at all. The therapist's first invitation is to Segun, not to Amara: "Segun, when your mother and father talk about this, what happens in the house?" Segun

says: "Mummy cries in the kitchen and Daddy's phone goes quiet." The sentence reorganises the session, and the therapist's harvest question is simply "did everyone hear that?"

The model's decision. The first invitation went to the quietest member, because the model's premise is that the family's truth is distributed and the therapist's task is to make it speakable. The choice was not neutral: it privileged the child's observation over the parents' explanation.

Session two. The therapist begins with the temperature reading: appreciations first. Tunde's appreciation of his wife — "she has carried this alone" — is the first warm sentence he has said in the family's hearing. Funke cries. The therapist slows down, does not speak for forty seconds, and then asks Funke what she would want to say to her husband if the phone were not between them.

The enactment. Funke: "I want to say that I am angry with you, and I do not know how to say it because you are doing the job for us." Tunde: silence, then, "I did not know." The therapist asks him to say it again to his wife rather than to the room, and he does. The harvest: Amara says "that is the first time I have heard you two talk without shouting".

Session three. The sculpture. Amara places her mother beside her, her father at the door of the room, Segun behind the father, and herself facing a window. In the reading, Tunde says "I am at the door because I am always about to leave". Funke says "I am beside her because if I am not, she disappears". Amara says "I am looking out of the window because that is where I want to be". The therapist asks Amara to change one thing; she moves her father from the door to a chair near the window, and says "he can sit there when he is home".

Session four. The translation: what would have to happen at home for the sentence "I am angry with you and I am saying it" to be possible? The family's answer is a weekly twenty-minute conversation on Sunday evenings, no phones, on the balcony, with the children free to join. The therapist resists improving the plan. Amara returns to school in week seven of the therapy. The family continues for four more sessions.

What the case teaches. That the model's interventions are small and precisely timed; that the therapist's main tool was pace, not technique; that a child's sentence can carry more information than two adults' accounts; and that a plan owned by the family is more likely to survive than a plan designed by the therapist.

7.6a The same family, with the therapy done badly

A second version, for contrast, with the errors marked in the margin.

Session one, badly. The therapist takes a forty-minute history of the school refusal, asks the parents to describe the family of origin, and concludes with a formulation: "Amara's symptom is a way of keeping the family together while her father is away." Amara is not asked a question. Segun is not addressed. Tunde, on speaker, is asked to "reflect on his absence".

Errors. The history before contact; the formulation delivered rather than co-constructed; the child described rather than asked; the father shamed, which in a family already organised around shame guarantees defensive withdrawal; and a systemic hypothesis offered without a single circular question.

Session two, badly. The therapist attempts a sculpture in the second meeting, directing the arrangement herself. Amara refuses to move, and the therapist interprets the refusal as resistance: "you are showing us how hard it is to let anyone near you." Amara stops speaking for the rest of the session.

Errors. The technique before the relationship; the therapist sculpting for the family; the interpretation of refusal as pathology rather than as information about the pace; and the absence of a harvest question in which the family could have told the therapist what the moment was like.

Session three, badly. The therapist asks Tunde, on speaker, "when did you last cry?" Tunde says nothing for a long time and then ends the call. Funke apologises. The therapist explains the importance of emotional expression and the family does not return.

Errors. The demand for feeling without contact or safety; the question asked in front of the children and down a telephone line; the lecture after the withdrawal; and no repair attempt.

The lesson in one sentence. In this model, the therapist's timing is the technique, and a correct intervention at the wrong moment is an error of the same magnitude as a wrong intervention.

7.7 Annotated transcript: one enactment, minute by minute

Sixteen turns from session two of the Adeyemi case. The transcript is short because the model's work is slow.

> **T1 Therapist:** Before we start — Funke, you said on the phone that this has been a hard week.
> **T2 Funke:** Segun was ill, and Tunde could not come, and Amara would not come out of her room.
> **T3 Therapist:** (pause) When you say "Tunde could not come" — what happens in your body as you say it?
> **T4 Funke:** I get tight. Here. (touches chest)
> **T5 Therapist:** Stay with that for a moment. What is the tightness?
> **T6 Funke:** It is anger. I am angry.
> **T7 Therapist:** And where does the anger go when it is tight like that?
> **T8 Funke:** Nowhere. It goes into the cooking.
> **T9 Therapist:** (to Tunde, on speaker) Tunde, did you hear what your wife just said?

> **T10 Tunde:** I heard.
> **T11 Therapist:** She said the anger goes into the cooking. What is it like for you to hear that?
> **T12 Tunde:** (long pause) I do not know what to do with it.
> **T13 Therapist:** I am not asking you to do anything with it yet. I am asking whether you can say something to her about hearing it.
> **T14 Tunde:** (pause) Funke. I am sorry that you are carrying it alone.
> **T15 Therapist:** (to the room, quietly) Let us stay here for a moment. Nobody needs to say anything.
> **T16 Therapist:** Funke, what happened when he said that?

Annotations. T1 is contact: one member is asked about their week before the problem is discussed, and the choice of member is deliberate — the mother is the family's manager and has not been met as a person.

T3 is the model's central intervention: the move from narrative to the body. The question does not ask what she felt then; it asks what happens in her body now, as she says it. T4 is the answer the model is looking for — a sensation rather than a theory.

T5–T6 is the discipline of staying with a sensation rather than moving to its explanation. The therapist does not say "so you feel angry", which would close the exploration; she asks what the tightness is, and the client names it herself.

T8 is the session's most valuable sentence, and it was not planned. "It goes into the cooking" is a precise description of a family process — anger converted into service — in the client's own idiom, and the therapist immediately makes it public within the family (T9), which is a Milan move (Week 6) deployed inside an experiential session.

T11–T13 is the enactment's grammar: notice, invite, hold. The therapist declines to make the husband responsible for the emotion (T13), which lowers the threat enough for him to speak (T14). The apology is short, awkward and effective, in the way that genuine contact usually is.

T15 is pace: the therapist protects the moment from being filled. Forty seconds of silence in a family session feels like five minutes to a trainee, and that discomfort is precisely the thing to be trained out.

T16 is the harvest: the moment is converted into something the family can describe, and the description is what they will remember.

What the trainee should notice. The transcript contains no interpretation, one piece of information about the therapist's inner state (none at all, in fact), and four questions that are about the present. The technique here is almost entirely restraint, and restraint is the hardest thing in this book to teach.

7.8 Laboratory: the four-move enactment

Sixty minutes, triads, one technique drilled until it is automatic.

Setup. A peer plays a family member describing a recent grievance; a second peer observes; the therapist practises the four moves — notice, invite, hold, harvest — with a different content each round. Three rounds of eight minutes, then rotate.

The observation sheet. For each round the observer records: the noticing sentence (verbatim); the invitation (verbatim, and whether it was an invitation or a demand); how long the therapist held silence (seconds, counted); and the harvest question.

The three rules of the drill. The noticing must refer to something observable — breath, jaw, hands, gaze, posture — and not to a feeling the therapist has inferred. The invitation must be answerable with no. The silence must be at least ten seconds, timed.

Marking rubric.

Criterion	Weight	Full marks
Noticing	25%	Concrete, behavioural, offered tentatively and not as diagnosis
Invitation	25%	Small, answerable, about contact rather than about content or advice
Holding	25%	At least ten seconds of silence after the invitation; no filling, no rescuing
Harvesting	25%	The moment converted into the family's own description afterwards

Debrief questions. Which move was hardest, and why? What did you feel in the silence, and what did you do with that feeling? What would you have said if the invitation had been refused?

The refusal rehearsal. Half the pairs practise refusal deliberately: the client says "no, I do not want to talk about that", and the therapist must respond without persuasion or withdrawal. The good response is something like: "that is fair. What is it like to be asked?" The rehearsal exists because refusals are common and trainees handle them by pushing; a therapist who can stay warm through a refusal has acquired the model's most portable skill.

7.9 Six errors in experiential work

1. **Enactment without a frame.** Inviting feeling before the family has a shared understanding of what the session is for, and without a plan for containment. Fix: contact first, and a stated ending time.
2. **The demand disguised as an invitation.** "Tell her how you feel" said in a tone that makes refusal impossible. Fix: the invitation is answerable with no, and the no is worked with.
3. **Flooding without containment.** A session in which a member sobs for twenty minutes and leaves raw. Fix: the model's own discipline — slow the pace, ground the body, end settled, plan the next contact.
4. **The therapist's disclosure as the main event.** Sessions in which the therapist's feelings become the material. Fix: brief, relational, in the service of the family's work, and examined in supervision.

5. **Charisma mistaken for effectiveness.** Families who admire the therapist and do not change. Fix: outcome measurement, and supervision that asks about dependency.
6. **The anti-technical stance as an excuse.** "I work with whatever comes up" used to avoid formulation, planning or evaluation. Fix: a written formulation after session three, and a review at every sixth.

7.10 Culture, faith and the model's critique

Where the model travels well. Its emphasis on the person, on warmth and on the family's own language is congenial in cultures where relationship precedes task. Nigerian and other African family work often begins with greeting, food, and the recognition of each person's standing, and an experiential therapist can work with that structure rather than against it. The model's insistence that the therapist is a person, not a technician, also fits contexts where the therapist is expected to be a human being with a life, a faith and a family.

Where it strains. Three places.

The expression of feeling as an unquestioned good. In many families, restraint is a virtue and public emotional display is a loss of dignity, particularly for men and for elders. A therapist who treats expression as health will pathologise a family's competence. The adjustment is not to abandon the model but to ask what expression means in this family, and to accept a smaller, more private form of contact.

The individual self as the unit of self-worth. Satir's language of self-worth, autonomy and the "five freedoms" carries a particular cultural assumption about the individuated self. Families organised around duty, hierarchy and interdependence are not thereby deficient, and a therapist who uses "congruence" to mean "say what you want regardless of the cost to the family" has imported a value and called it health.

The elder's authority. Enactment between a young therapist and a senior family member can be read as disrespect. The model's answer is that the therapist addresses the elder with formal respect and enquires about the younger members' experience, rather than inviting the elder to be confronted.

Faith and the experiential model. The model is hospitable to spiritual experience because it takes feeling and symbol seriously. A family's prayer, its sense of a presence in the room, its grief rituals, can all be worked with as experience rather than as belief. The caution is symmetrical: the therapist must not use the family's faith as an emotional technique, and must not substitute their own spiritual experience for the family's.

> **DEBATE**
> "Satir's 'five freedoms' are a Western liberal anthropology dressed as universal health." Argue both sides, then say what you would keep.

7.10a The two strongest critiques, and the model's answers

Critique one: it cannot be manualised, and therefore cannot be tested. The intervention is the therapist's person, and the person varies. Two therapists trained in the same institute will conduct visibly different sessions, and the model has no specification against which fidelity could be measured. To a research funder this is close to a disqualification.

The model's answer. Manualisation is one route to evidence, not the only one; common-factor research (Week 11) shows that the therapist's alliance and qualities account for substantial variance, which is the model's central claim; and the model's techniques can be specified even where the stance cannot. Family sculpture, the temperature reading and the four-move enactment are all describable, trainable and measurable.

The fair verdict. The critique is broadly right about the package and wrong about the components. The model should be taught as a stance with a small set of specifiable techniques, and evaluated as such.

Critique two: it is unsafe for families in which expression has been dangerous. A family with a violent member, a family in which one person's emotions have historically governed the household, a family in which a child has been scapegoated — in each, an invitation to express feeling can produce harm, and the model's literature gives little guidance on screening.

The model's answer. The original texts assume voluntary, non-violent families and a therapist with the skill to contain; where those conditions are absent, the model's own emphasis on safety and pace should govern, and the therapist should not invite material they cannot hold. The honest verdict is that this answer is insufficient: the model needs a screening protocol, and it does not have one.

What a contemporary practitioner should add. A standard pre-therapy screen for violence and coercion, a containment plan written before the first enactment, and a rule that no enactment is attempted in a session where the therapist has reason to believe a member will be harmed for what is said, at home or afterwards. That rule is not in the model's canon; it is a professional obligation that overrides the technique, and it should be written into the service's policy as well as the therapist's practice.

7.11 Evidence note: what is known and what is claimed

The evidence base. Experiential family therapy, in Satir's and Whitaker's forms, has very little randomised trial evidence. The classic literature is case-based and theoretical. Satir's approach has been studied mainly through training outcome research and qualitative work; Whitaker's symbolic-experiential therapy likewise.

What has been inherited and tested. The model's central mechanisms have been absorbed into emotionally focused therapy (MFT 709) and into common-factor research: emotional engagement in session, therapeutic presence, the processing of affect, and enactment are all studied constructs. EFT has a substantial trial literature, and its

manuals cite the experiential tradition as an ancestor. The evidence for those mechanisms therefore supports the model's components without supporting the model as a package.

What the matrix cell should say. "No RCT evidence for Satir or Whitaker as packages; the model's mechanisms (affect regulation in session, enactment, therapeutic presence) are supported through EFT and common-factor research; the techniques are teachable and observable; the therapist-variable problem is the central limitation."

The practitioner's claim, in a defensible form. "I use the model's discipline of contact, pace and enactment because it deepens the session's emotional processing, and I measure outcomes and alliance because the model's evidence base does not tell me whether I am effective with this family." That sentence is what a viva examiner will want, and it is honest about the gap.

7.12 Key terms

Term	Definition
Congruence	Words, feeling and body agreeing; speaking for oneself
Incongruent stance	Placating, blaming, super-reasonable or irrelevant; a strategy that protects the relationship by hiding the truth
Self-worth	Satir's organising variable; the capacity to bear the risk of speaking truly
Family sculpture	Physical arrangement of the family representing the emotional map, then rearranged
Enactment	Bringing the relationship into the present session and letting it happen
Temperature reading	Five-part structured check-in: appreciations, information, puzzles, complaints, wishes
Parts party	Naming and voicing internal parts in tension
Family reconstruction	Satir's guided three-generation enactment method
Symbolic-experiential	Whitaker's term for therapy that works through symbol and experience
Use of self	The therapist's own presence, feeling and spontaneity as the instrument
Pace	The deliberate slowing of the session; the model's core discipline
Containment	The frame that makes emotional intensity safe and endable

7.13 Self-test

1. Name the five stances and the function they share.
2. What is the difference between an invitation and a demand, and how is the difference tested?
3. Give the four moves of an enactment and the sentence that performs each.
4. Why is the harvest the step trainees skip, and what does it produce?
5. What is the model's central evidence problem?
6. State the two strongest critiques and the fairest verdict on each.

7. What does "use of self" require of the therapist professionally?
8. Name three adaptations for a family in which emotional restraint is a value.

Answers. 1. Placating, blaming, super-reasonable, irrelevant, congruent; the first four protect the relationship by hiding the truth, and the fifth risks the truth for the sake of contact. 2. An invitation can be answered with no without consequence to the relationship; it is tested by the therapist's response to a refusal — no persuasion, no withdrawal, curiosity about the refusal. 3. Notice ("I see your jaw tighten when she says that"); invite ("can you say that to her rather than about her?"); hold ("stay with it a moment"); harvest ("what happened in the room just then?"). 4. Because it feels like an anticlimax after intensity; it converts the moment into the family's own description, which is what carries the learning out of the room. 5. No manualisation and no RCTs, so the package cannot be tested or fidelity-checked, while its mechanisms are supported through EFT and common-factor research. 6. Critique one — untestable package; fairest verdict, right about the package, wrong about the components. Critique two — unsafe where expression has been dangerous; fairest verdict, the model's answer is insufficient and a screening protocol must be added. 7. It requires supervision of the therapist's own material, outcome measurement against dependency, and a caseload that includes non-experiential work. 8. Any three of: ask what expression means in this family; accept a smaller private form of contact; avoid confrontation of elders; use enactment in pairs rather than the whole family; use writing or drawing instead of speech for the hardest material.

7.14 Reflective task

Think of a moment in your own family when somebody said something true and the room changed. Write it in three paragraphs: what was said; what happened in the bodies of the people present; and what the family did with it afterwards. Do not analyse it and do not draw a conclusion. Bring it to the seminar, and notice which paragraph was hardest to write — that is usually the one that tells you something about your own practice. If nothing was hard to write, write about that instead, and be suspicious of it.

7.15 Readings for this week

- **Satir (1972)**, Peoplemaking, the accessible statement of the stances and self-worth, read for the vocabulary rather than the theory.
- **Satir & Baldwin (1983)**, Satir Step by Step, a session-by-session account, the best source for the techniques in action.
- **Whitaker & Keith (1981)**, "Symbolic-experiential family therapy", in Gurman & Kniskern, for the therapist's use of self and the model's attitude to technique.
- **Napier & Whitaker (1978)**, The Family Crucible, the model's most readable casebook and the book most students remember.
- **Bitter (1988)**, "Family mapping and family sculpture", for the technique described systematically.

- **Greenberg & Johnson (1988)**, Emotionally Focused Therapy for Couples, read the opening chapters for how the tradition became manualised and testable — the bridge to MFT 709.

Read The Family Crucible as a novel and then again as a technique manual; the second reading is the one that shows how much of the therapy is timing. Keep a note of every silence in the transcripts you read, and count them. For the essay, cite Satir and Whitaker for the model, Greenberg and Johnson for the mechanism, and the common-factor literature for the therapist variable, and be explicit that the package has no trial evidence. The reading log question for this week is different from last week's: not "what does the argument assume?" but "what would this therapist have to be like for this to work?" Answer it in five sentences and bring the answer to supervision, because it is the question the model asks of everyone who practises it.

Next: Week 8 — Cognitive-behavioural and behavioural couple therapy

---

Week 8 — Cognitive-behavioural and behavioural couple therapy

The question. What behaviours and beliefs maintain the distress, and how can they be changed directly?

Core concepts. Behavioural exchange and communication training (TBCT); cognitive restructuring of relationship standards and attributions; the role of selective attention (noticing negatives and missing positives); homework as the engine of change; skills deficit versus skills performance.

Therapy. Structured, time-limited, with agenda and homework. Skills are taught and rehearsed in the room, then practised between sessions. Integrative behavioural couples therapy addresses both behaviour and acceptance, with attention to culture and values.

Signature techniques.

- **Behavioural exchange:** each partner lists small, achievable caring behaviours; each does them daily.
- **Communication training:** speaker–listener with a talking object (see Figure 35, MFT 716).
- **Attribution retraining:** "When he is late, what do you assume? What else could it mean?"
- **Problem-solving:** define one problem, generate options, choose, trial, review.

Evidence. Among the strongest in the field; effective for relationship distress, with good evidence for depression treated in a couple format.

Blind spots. Can feel mechanistic; may under-weight emotion and attachment; assumes a degree of motivation and literacy; "skills" can be performed in session and abandoned at home.

PRACTICAL

With a peer, design a behavioural exchange contract: three behaviours each, defined observably, cheap, daily, and mutually agreeable. Then critique it: what could make this fail in a real family within two weeks?

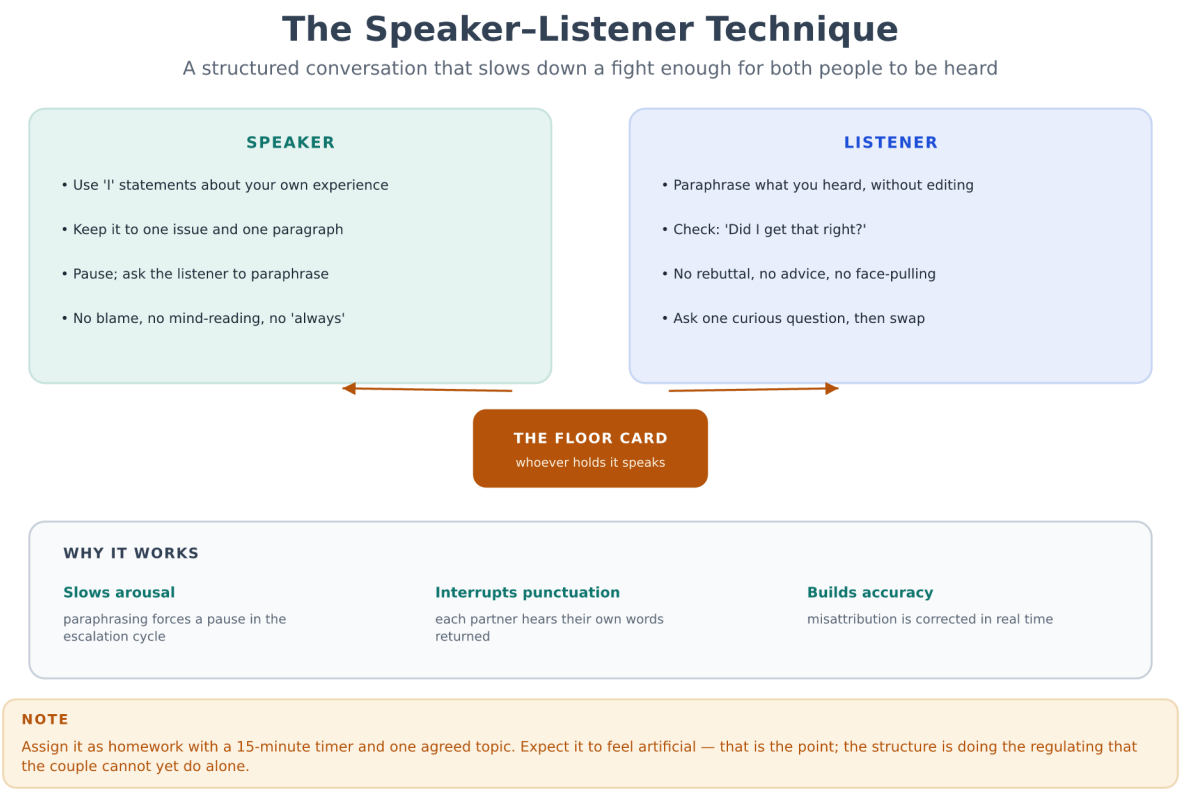


Figure 8.1 — Speaker–listener structure: the turn-taking frame that behavioural couple therapy borrows and formalises

8.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	From insight to behaviour	Why the behavioural turn happened, and what it gave up	Name one thing every previous model left to chance
10–25	The behavioural exchange	Two lists, one week, and the arithmetic of small positives	Draft a three-item exchange contract
25–40	Communication training	Model speaker–listener with a volunteer; break it deliberately	Run four turns with a talking object
40–55	Attribution and standards	Where the distress actually lives: in the interpretation	Re-attribute three real complaints
55–70	Problem-solving	The six steps, demonstrated on a live problem	Solve one problem in six minutes
70–82	IBCT: acceptance beside change	When change strategies fail and what replaces them	Rewrite one demand as a soft disclosure
82–90	Evidence and limits	The strongest evidence base in the course, and its blind spots	Fill the matrix row

The teaching decision that makes the week work. Behavioural couple therapy is the easiest model in this book to demonstrate and the hardest to demonstrate well. A lecturer who shows speaker–listener badly makes the model look absurd; a lecturer who shows it well makes it look easy and then has to explain why trainees find it hard. The solution is to demonstrate it twice: once as a caricature (rigid, mechanical, with the therapist policing the rules), and once as it is meant to be (warm, brief, with the therapist tracking the couple's emotional response to the structure). The class then has two reference points, and the difference between them is the week's content.

What the lecture should establish early. That the model is not a denial of feeling. Behavioural couple therapy works because behaviour and attribution carry feeling; a couple who exchange three small acts of care daily are not being trained like dogs, they are being given evidence against a belief that nothing will ever change. The model's logic is that the evidence comes first and the feeling follows, which is the inverse of Week 7's logic — and the two models are best taught against each other.

8.2 Core concepts

The behavioural premise. Distress is maintained by observable behaviour and by the interpretations that accompany it, and both can be changed directly. The model does not deny history, attachment or meaning; it declines to work on them first. The claim is strategic: change the daily exchanges and the attributions, and the emotional climate moves with them.

Behavioural exchange (the "give-get" or reciprocity model). Each partner identifies small, frequent, low-cost behaviours that the other would find pleasing, and performs them at a higher rate. The mechanism is reciprocal reinforcement: positive behaviour increases the other's positive behaviour, and the couple begins to experience the relationship as rewarding rather than only as effortful.

The arithmetic that makes it work. The behaviours must be small (a phone call at lunchtime, not a holiday), frequent (daily, not monthly), cheap (no cost to the giver's scarce resources), observable (a described act, not an attitude), and agreeable to both. A list of three such behaviours per partner, performed for two weeks, changes the ratio of positive to negative exchanges more than a single grand gesture, because the ratio is what the couple's attention is monitoring.

Selective attention and the negative ratio. Distressed couples notice negatives and miss positives; the model's term is selective attention, and its consequence is that the couple's account of the relationship becomes an account of injuries. The therapy's first task is often perceptual rather than behavioural: teach each partner to see what is actually being done, before asking for anything new.

Communication training (TBCT). Traditional behavioural couple therapy pairs behavioural exchange with training in communication and problem-solving. The couple learns a structure: one speaks, one listens and reflects, then they swap; problems are defined, options generated, one chosen, a trial agreed, and a review scheduled. The

structure is deliberately artificial, and the artificiality is the point — it interrupts the couple's established escalation long enough for a different pattern to be possible.

Attribution retraining. The cognitive half of the model. Distress is carried less by events than by the explanations attached to them: "he is late because he does not care" versus "he is late because the traffic on that road is impossible and he hates being late". The therapy does not ask the couple to be positive; it asks them to generate alternative explanations and then to test them, which is an evidential procedure rather than an exercise in optimism.

Relationship standards. The cognitive model's deeper layer: each partner holds standards about what a marriage should be — about intimacy, fairness, autonomy, loyalty, communication — and most of the couple's conflict is a collision of standards rather than a failure of skill. The therapeutic move is to make the standards explicit ("what should a husband do when his wife is upset?") and then to negotiate the difference rather than to adjudicate who is right.

Skills deficit versus skills performance. A distinction that determines the treatment. A couple who cannot listen because they have never been taught a structure has a deficit; a couple who can listen perfectly in the session and never at home has a performance problem. Deficits are treated with training; performance problems are treated by examining what happens at home that makes the skill unavailable — usually an emotional or contextual variable the behavioural model must then import.

Homework as the engine. The model's central delivery mechanism. Change happens between sessions, and the session's task is to design, rehearse and review the homework. Consequently the session has an agenda, the homework is written down, the obstacles are predicted, and the review comes first at the next meeting. A behavioural therapist without homework is not doing the model.

Integrative behavioural couple therapy (IBCT). Christensen and Jacobson's revision, which added acceptance to change. Its premise: some differences are not problems to be solved but incompatibilities to be tolerated, and the attempt to change them produces the distress. IBCT uses empathic joining around the difference (each partner's behaviour is described as understandable given its history and its function), unified detachment (the couple examines the pattern together rather than each other), and tolerance building (through rehearsal and the deliberate, controlled expression of the difference).

The DEEP analysis. IBCT's formulation tool: Differences (what actually differs), Emotional sensitivity (what each partner feels when the difference is activated), External stress (contexts that make it worse), and Pattern (the dance that has formed around it). A formulation in four columns, and a useful one to teach because it can be completed in a single session.

8.2a Two behavioural ideas, misread and corrected

Misreading one: "the model is about being nice." Trainees often reduce behavioural exchange to pleasantness, and produce contracts that read like greeting-card instructions: "be more affectionate", "show appreciation", "make time for us". Such items fail because they are unobservable and unverifiable, and because they ask for a mood rather than an act. The correction is the specificity test — would a camera record it? — applied to every item on the list.

The corrected version. Not "be more affectionate" but "put your hand on my shoulder when you pass me in the kitchen"; not "show appreciation" but "say thank you when I cook, even if you do not like it"; not "make time for us" but "twenty minutes on the balcony after the children are asleep, on Thursdays, no phones". Each item is filmable, cheap, daily and chosen by the person who receives it.

Misreading two: "attribution retraining is positive thinking." The technique is not an instruction to see the best in one's partner, which would be a demand for self-deception. It is a demand for evidence: an event has several possible explanations, a distressed couple habitually selects the most hostile, and the therapist's task is to restore the full set and then to test which one fits.

The corrected version, in four steps. Name the event ("he came home at ten and did not call"). Name the automatic explanation ("he does not think about me"). Generate three alternatives that are not hostile and not implausible ("the meeting overran"; "his phone died"; "he was avoiding a conversation he dreads"). Then test: what would each explanation predict, and what actually happened? The couple usually discovers that they have been running a hostile hypothesis without ever checking it, and the discovery is more persuasive than any reassurance the therapist could offer.

The pair of corrections, stated as a rule. Specificity for behaviour, evidence for cognition. Both are disciplines rather than attitudes, and both are teachable in a session — which is why this model has the strongest training literature in the field.

8.3 The therapy: a structured course, session by session

The model's shape is unlike anything in Weeks 2–7: it is a course with a syllabus.

Session one — assessment and formulation. Both partners are seen together for most of the session, with a short individual segment each (and a stated policy on secrets, taken from MFT 701 Week 6). The therapist takes a behavioural history of the presenting complaint: what happens, when, who does what first, what follows. The DEEP analysis is completed. Two or three questionnaires are administered — a satisfaction measure and, where depression is present, a mood measure — and the scores are shown to the couple, which is itself an intervention.

Session two — the rationale and the first exchange. The formulation is presented in plain language, the treatment is described, and the first behavioural exchange is negotiated. The couple leaves with a written list of three behaviours each and a chart.

Sessions three to six — skills. Speaker–listener, then problem-solving, then attribution work. Each session begins with the homework review, which occupies a third of the time;

each ends with a rehearsal of the next week's task in the room. The therapist teaches by demonstration, then by coaching, then by observing the couple perform it without help.

Sessions seven to ten — integration and consolidation. The couple brings a real problem and runs the whole procedure while the therapist stays quiet. Attribution work moves to the couple's core standards. Relapse is planned: what the couple will do when the old pattern returns, and how they will know.

The typical course. Twelve to twenty sessions for relationship distress; eight to twelve for a focused problem; and a planned ending with a review at three and six months. The model's brevity is a design feature, and the therapist's discipline is to end on time.

The therapeutic relationship in a structured model. The alliance matters here as much as anywhere, and the structure can damage it if the therapist becomes a referee. Three disciplines protect the alliance: the agenda is agreed with the couple rather than imposed; the therapist names feeling when the structure produces it ("that was hard to hear; stay with it for a moment"); and the therapist does not adjudicate whose account of an event is true, only whether the process between them is working.

What the therapist does when the couple will not do homework. First, assume the task was badly designed rather than that the couple is resistant. Second, review the obstacle concretely: what happened at the moment the task was to be done? Third, shrink the task until it is almost trivial, and rebuild. Fourth, if the pattern persists across three tasks, the formulation is wrong: something is being protected, and the model must import the emotional or systemic question it usually defers.

8.3a The first three sessions in detail, with the words

Session one, minute by minute.

Minutes 0–5. The frame. "We will meet for fifty minutes. I will ask each of you about the problem and about what you have already tried, I will ask you to fill in two short questionnaires, and at the end I will tell you what I think and what I would suggest. I will also see each of you alone for ten minutes, and I will tell you both what the rule about secrets is before we start: if either of you tells me something that affects the other, I will ask you to say it in the room, and I will not carry it for you."

Minutes 5–20. The behavioural history. Not "how long have you been unhappy?" but "take last Tuesday. What happened from the moment you both got home?" The therapist writes a timeline, and marks the points at which each partner's behaviour changed. Two or three such days give more information than a year of narrative.

Minutes 20–30. Individual segments. Each partner is asked two questions: what they would want the other to know, and what they would not want said in the room. The second question is the one that produces the material for the formulation, and the therapist's use of it is the ethical test of the model.

Minutes 30–40. Questionnaires. Satisfaction and mood. The therapist scores them in the room and says the numbers aloud. Couples are frequently startled by the gap between their two scores, and the gap is a finding the therapy will work with.

Minutes 40–50. The formulation, offered and tested. "Here is what I think is happening. You are both working hard and both feeling unappreciated, and the reason is that the things each of you does to show care are not the things the other notices. When that happens for long enough, each of you begins to explain the other's behaviour as indifference. I may be wrong; tell me what fits and what does not."

Session two, the first exchange. The therapist asks each partner for three behaviours — small, daily, cheap, observable — that the other would find pleasing. The negotiation is explicit: each item is read aloud, the receiving partner says whether it would help, and items that are too big or too vague are reduced on the spot. The list is written by hand, in duplicate, and both copies go home.

The mistake to avoid. Accepting a list of items that the giver will find easy rather than items the receiver will find meaningful. The technique's logic requires the receiver's preference to govern, and the therapist must enforce it or the exchange becomes another occasion for disappointment.

Session three, the first skills session. Speaker–listener is introduced with a talking object, in three steps: the therapist demonstrates with one partner while the other observes; the couple performs four turns with the therapist coaching; the couple performs four turns without coaching while the therapist stays silent and takes notes. The debrief asks what the listener found hard, which is where the emotional material enters and where the model must decide whether it can stay behavioural.

8.4 Techniques, specified

Behavioural exchange contract. Three items each; written; daily; reviewed weekly; revised when an item fails. The contract is not a scoreboard and must not become one — the moment a couple begins to audit each other's compliance, the technique has inverted, and the therapist must step in.

Speaker–listener. Rules: the speaker speaks for themselves, in short turns, about one topic; the listener does not rebut, defend or advise, but reflects back what they heard and checks it; then they swap. The talking object makes the rule physical, which matters more than it sounds — the object carries the turn, so neither partner has to concede authority.

The empathic reflection step. After reflecting content, the listener is asked to guess the feeling: "and I think you felt dismissed when I said that". Guessing is permitted and is often wrong; the speaker corrects. This step is where behavioural communication training and emotion-focused work meet, and it is the step most often dropped by trainees who are anxious about time.

Attribution retraining in four moves. Event, automatic explanation, three alternatives, test. Conducted in the room, with both partners contributing alternatives for each other's behaviour, which produces the unusual experience of hearing one's partner generate a charitable explanation aloud.

Standards clarification. "What should a wife do when her husband is silent?" "What does a good marriage look like in your family, and in the family you grew up in?" The two

answers are compared, and the difference between the partners' standards — not between either standard and the therapist's — becomes the subject.

Problem-solving, six steps. Define one problem in behavioural terms; brainstorm options without evaluation; evaluate; choose one; agree a trial period and a review date; review. The discipline is the separation of generation from evaluation, which couples in distress cannot do without a structure.

Positive exchange of the week. A lighter variant for couples who find the contract aversive: each partner names one thing the other did that they appreciated, at the start of each session, for four weeks. It is the temperature reading's first box (Week 7) imported into a behavioural frame, and it is the single cheapest intervention in this book.

The relapse plan. Written in the penultimate session: the early warning signs the couple now recognises, the two behaviours they will reinstate first, and the point at which they will ask for help again.

8.5 The first session with a couple who have stopped speaking

The model's most common presentation in practice is not a couple who fight but a couple who have gone quiet, and the structured approach has to be adapted.

The adaptation. The behavioural history is taken as a history of absences: what used to happen that no longer does; when the last ordinary conversation took place; what each partner does in the evening. The question "what did you do together in the first year?" is asked in session one, because it establishes that the behaviour exists and has simply stopped, which is a more hopeful formulation than a deficiency.

The first exchange, for a silent couple. Items are chosen for their minimal emotional cost: leaving a note, making a second cup of tea, sending a message from work about something trivial. The point is not romance but the re-establishment of a channel with a low threshold. A couple who cannot yet speak can usually exchange small acts, and the acts make speech easier.

The first skills session, for a silent couple. The talking object is introduced without reference to the problem, using a neutral topic ("plan the weekend"), so that the structure can be learned before it is asked to carry pain. Trainees find this counter-intuitive and it is the technique's most useful application.

The question that must be asked in session one. "Is there anyone in this relationship who is afraid?" Asked of each partner separately, in the individual segment, and recorded. The behavioural model's evidence base assumes non-violent couples, and a therapist who begins a communication exercise with a couple in which one partner is afraid has created a structured opportunity for coercion.

8.5a A behavioural exchange contract, designed and then criticised

The week's practical, worked fully so that the seminar can critique a real artefact.

The couple. The Ogundipes: mother 44, father 47, married nineteen years, two children, presenting with "we have nothing to say to each other and everything I do is wrong".

The draft contract.

Giver	Behaviour	Frequency
Husband	Sends a message from work at lunchtime, no content required	Weekdays
Husband	Makes the morning tea and leaves it by her side of the bed	Daily
Husband	Watches one episode of her programme with her without his phone	Twice a week
Wife	Says thank you when he does something around the house	Daily
Wife	Asks him one question about his work at dinner	Weekdays
Wife	Leaves his work clothes out on Sunday evening	Weekly

The critique, in the way the seminar should conduct it.

Item one is well specified and low cost, and it establishes a channel. Risk: it becomes a compliance test — if he misses a day, she has evidence of indifference. The therapist's counter: agree in advance that missed days are expected and are mentioned without commentary.

Item two is filmable and cheap, and it is chosen by the receiver. Risk: it is intimate in a way the couple may not currently tolerate; the husband may feel foolish. Counter: rehearse it in the session — literally, with a cup — so the awkwardness is spent in the room.

Item three asks for a change in his attention, not his behaviour, and it contains a hidden criticism ("and without his phone"). It will be experienced as a reproach each time it is performed. Better: "we watch one episode together on Tuesday and Friday, and the phone stays in the kitchen." The reformulation moves the instruction from him to the situation.

Item four is a courtesy that she already performs — she chose it because it is easy, which is exactly the error identified in 8.3a. It will not change his experience. Better: "asks him, once a week, what he would like for dinner and cooks it."

Item five is good: it is small, daily and it opens a channel that has closed. Risk: it is an interview, and he may answer in monosyllables; the contract should specify that her question is the whole behaviour, not his answer.

Item six is a service behaviour with a history of resentment — "I do everything for him" is part of the couple's complaint. Chosen by her, acceptable to him, and it risks confirming the pattern. A better item would be chosen by him from a list she generates: what would he actually notice?

The three general lessons. Items must be chosen by the receiver; items must be about situations as well as persons; and any item that carries a criticism will be heard as one every time it is performed. A contract that survives these three tests will usually produce measurable change within three weeks, and the measurement is the couple's own count, not the therapist's impression.

8.6 Case: the Adeyemis, in behavioural form

The recurring family, seen through this model, to make the translation from systemic to behavioural explicit.

The presentation. Amara Adeyemi (14) has refused school for five weeks. In a behavioural formulation the presenting complaint is decomposed: what Amara does at 7 a.m. (stays in bed, says she feels sick); what her mother does (asks three times, then telephones the school, then stays home); what her father does by telephone (instructs, threatens, disengages); what the school does (marks the absence, telephones at 10 a.m.). The pattern is a loop of four elements, and each element is observable.

The behavioural formulation. Amara's avoidance is negatively reinforced: staying in bed removes the aversive stimulus (school). Her mother's repeated asking is reinforced by the occasional success and extinguished by the usual failure, which produces escalation. The father's telephone instructions are reinforced by the relief of ending the call. Nobody's behaviour is irrational; every element is maintained by its consequence.

The plan, in behavioural terms. A graded return: three half-days in week one, four in week two, full attendance by week three, with a specified morning routine and a written script for the mother (one instruction, one consequence, no repetition), and a scheduled daily telephone call from the father at a fixed time rather than on demand. The parents' behaviour is the target, not Amara's, and the therapist says so.

What this model adds that the structural and Milanese weeks did not. Specificity of the morning, a measurable outcome (half-days attended), a written script, and a review date. What it loses: the meaning of the refusal, which Week 6's hypothesising had opened — the therapist never asks what school has come to represent, and in this family the answer (a bullying incident the parents do not know about) only emerges because the mother mentions it in the third session.

The integration the student should note. The behavioural plan was necessary and insufficient: it produced attendance in three weeks, and the attendance collapsed in week six when the bullying re-emerged. The model's own literature acknowledges this — behavioural change without the emotional and contextual formulation is a repair without a diagnosis.

8.6a The same couple in a behavioural formulation, and in a systemic one

A side-by-side comparison, because the examination will ask for it and because the contrast teaches both models.

The couple. The Ogundipes: nineteen years married, two children, presenting with "nothing to say and everything I do is wrong".

In behavioural language. Two low rates of reinforcing behaviour; a high rate of aversive exchange; selective attention to negatives; a hostile attribution style ("he does it because he can"); the wife's standard is that a husband should initiate conversation, the husband's is that a wife should be content with provision; a deficit in the couple's ability to raise a complaint without escalation; and a performance problem in the skills they can demonstrate in the clinic. The treatment plan writes itself: exchange, speaker–listener,

attribution retraining, standards clarification, review at four weeks with a satisfaction measure.

In systemic language (Weeks 4–6). The couple's pattern is complementary and escalating: she pursues, he withdraws, she pursues harder, he withdraws further, and each punctuates the sequence at the other's move. The symptom (the silence) is stabilising: it protects the husband from a conversation about his business debts and protects the wife from a decision about her mother's care, and neither of these subjects has been mentioned in nineteen years. The wife's care for her own mother is described by the husband as "her real marriage". A Milanese hypothesis would be that the silence keeps two unspoken arrangements intact, and a structural intervention would work on the boundary between the couple and the wife's family of origin.

What each formulation makes possible. The behavioural formulation gives specific, measurable, time-limited actions and a review date. The systemic formulation gives the meaning of the silence and the reason it has survived, and it predicts that a skills-based intervention will improve the evenings and leave the marriage's central subject untouched.

What each formulation cannot see. The behavioural formulation, as written, contains no account of the wife's mother, the debts, or the nineteen years; it treats the presenting complaint as the problem. The systemic formulation contains no measurable action, no review date and no outcome measure; it can describe the system beautifully for six months without anything changing on a Tuesday evening.

The integration a competent therapist performs. Use the systemic formulation to choose the target, and the behavioural method to change it. Specifically: hypothesise that the silence protects a subject; ask one circular question about who would be most affected if the couple talked; and then build a behavioural sequence that makes the first conversation survivable — a topic, a time limit, a structure, and a review. That sequence is neither a pure contract nor a pure enactment; it is what integration looks like when it is done rather than declared.

The examination answer, in three sentences. "The behavioural model treats the exchange and the attribution; the systemic model treats the meaning and the function. For this couple, the meaning is a protected subject and the exchange is a rate of reinforcement; the therapy needs the second to reach the first. I would formulate systemically, intervene behaviourally, and measure both."

8.7 Annotated transcript: attribution retraining with a couple

Twelve turns, from session four with the Ogundipes. The therapist is working on one incident.

> **T1 Therapist:** Let us take one small thing from last week. You said he was late on Wednesday.
> **T2 Wife:** Forty minutes. He did not call.
> **T3 Therapist:** When the forty minutes had passed and the phone had not rung, what went through your mind?
> **T4 Wife:** That he was with his friends. That he does it because he can.
> **T5 Therapist:** And that thought — what did it do to you?
> **T6 Wife:** It made me cold. I did not speak to him when he came in.
> **T7 Therapist:** (to husband) What actually happened on Wednesday?
> **T8 Husband:** The generator at the office failed and I could not leave the till.
> **T9 Therapist:** (to wife) So the explanation you had was one of several. Let us see if we can find the others. What else could forty minutes and no call mean, in your husband's life as you know it?

> **T10 Wife:** (pause) The traffic on that road. Or his phone. Or — he might have been afraid to call and say he was late.
> **T11 Therapist:** (to husband) Which of those is closest?
> **T12 Husband:** The last one. I knew she would be angry, so I thought it was better to arrive than to call.

Annotations. T1–T2 establishes a small, recent, dated incident. Small incidents are used deliberately: the technique needs an event whose explanation can be examined without the whole marriage at stake.

T3 is the key question. It asks for the automatic thought, not for the feeling, and it asks it at the moment of the event ("when the forty minutes had passed"), which is where attributions live.

T4 is the hostile attribution, stated plainly and without correction. The therapist's discipline is to receive it without arguing — an argued attribution becomes a defended one.

T5–T6 links attribution to behaviour within four turns, which is the technique's logic: the thought produced a behaviour (coldness) that produced an outcome (a silent evening) that confirmed the thought.

T7–T8 brings the actual explanation into the room, and the order matters: the therapist hears the wife's attribution first, in full, before the husband's account is admitted. Hearing the husband first would have produced a correction; hearing the wife first produces a comparison.

T9 is the technique proper: "what else could it mean?" — addressed to the wife, who generated three alternatives including one she had not previously considered, and one of which turned out to be true.

T11–T12 closes the loop with evidence, and the husband's answer is the session's most useful sentence: he did not call because of her anticipated anger, which means the wife's

attribution (indifference) and the husband's motive (fear) were both present and only one was in her model of him. The therapist writes the sentence down.

What the trainee should notice. The therapist never says "you are being unfair" or "he was stuck at work". The technique works because the couple produce the alternatives and test them themselves; the therapist's only intervention of substance is the question at T9.

8.7a Speaker–listener, three ways it goes wrong

A taxonomy from recorded training sessions, with the correction for each.

Failure one — the reflecting machine. The listener repeats the speaker's words accurately and flatly, with no guess at feeling and no contact. The speaker reports feeling unheard despite the accuracy. The cause: the trainee has learned the form and not the function; reflection is a procedure for demonstrating attention, and attention is not established by word-matching. The correction: add the feeling guess, and make one deliberate error so the speaker has something to correct — being corrected is evidence of being listened to.

Failure two — the therapist's rule-policing. The therapist interrupts every thirty seconds: "you did not reflect"; "that was a rebuttal"; "swap now". The couple become pupils, the emotional content disappears, and the session becomes a lesson in which both partners are failing. The cause: the trainee's anxiety about fidelity, displaced onto the couple. The correction: coach silently for two minutes with notes, then debrief in one paragraph; the structure is a container, not a test.

Failure three — the disclosure that becomes an accusation. The speaker uses the structure to deliver a grievance with perfect form: "when you ignore me at your family's house I feel humiliated, and I need you to stop". The rules have been followed and the partner has been indicted. The cause: the model has no specification for the content of a turn, only its form. The correction: a prior step, in which the speaker is asked to describe one specific incident and one feeling rather than a pattern and a demand, and in which the therapist checks the sentence with the speaker before the turn begins.

The general lesson. Structure protects a couple from their own escalation, and it does not teach them what to say. That is why the behavioural model works best in combination with a formulation that knows what the conversation is for.

8.8 Laboratory: design, rehearse, review

Sixty minutes, pairs, three tasks in sequence.

Task one, the exchange contract (twenty minutes). Each student writes a three-item contract for a couple they are currently seeing (anonymised) or for a role-played couple played by their partner. The partner plays the receiver and applies the three tests: could a camera record it; is it daily; does it carry a criticism? Any item that fails is rewritten twice.

Task two, speaker–listener under coaching (twenty minutes). Four turns on a specified topic, with the observer using a checklist: speaker used "I"; listener reflected content; listener guessed feeling; no rebuttal; swap occurred at the object. The observer interrupts only when a rule is broken, and counts the interruptions.

Task three, the formulation in four columns (twenty minutes). Complete a DEEP analysis for the same couple: differences, emotional sensitivities, external stresses, pattern. Then write one sentence that presents it to the couple without blame.

Marking rubric.

Criterion	Weight	Full marks
Specificity of behaviours	30%	Every item filmable, daily, cheap, chosen by the receiver, free of criticism
Fidelity to speaker–listener	25%	Structure maintained under pressure; feeling reflection present; no therapist refereeing
Formulation quality	25%	Four columns complete; emotional sensitivities named without pathologising either partner
Review discipline	20%	Written homework, predicted obstacle, review date, outcome measure named

The debrief question that matters. Which task felt most like a technique and least like a relationship? The answer is usually the contract, and the discussion that follows is the week's real content: the model requires the therapist to hold a procedure and a relationship at the same time, and the procedure is easier to learn than the holding.

8.9 Six errors in behavioural couple work

1. **The contract as a scoreboard.** Compliance auditing replaces goodwill, and the couple begin to prosecute each other with the list. Fix: agree in advance that missed items are mentioned without commentary, and revise rather than enforce.
2. **Teaching skills to a couple who need safety.** Communication training in a relationship containing coercion gives the coercive partner a structured instrument. Fix: screen in session one (8.5), and do not run the exercise where fear is present.
3. **The therapist as referee.** Adjudicating whose account of an incident is true. Fix: work only on the process between them, and say so.
4. **Homework set but not designed.** A task assigned in the last five minutes, without rehearsal and without a predicted obstacle. Fix: rehearse in the room and write the obstacle down.
5. **Dropping the feeling.** A technically correct session in which nobody's experience was named. Fix: the empathic reflection step, used every time.
6. **Ignoring the standard.** Treating a collision of relationship standards as a skills deficit. Fix: the standards clarification, and IBCT's acceptance work where the difference is not solvable.

8.10 Culture, faith and the model's critique

Where the model travels well. Its structure, explicitness and time-limited frame are congenial to families who want to know what will happen and how long it will take; its behavioural items can be built from the family's own practices rather than from a therapist's idea of romance; and its problem-solving structure is teachable across literacy levels if it is done aloud rather than on paper.

Where it strains. First, the model's behavioural items are culturally loaded: "a phone call at lunchtime" presupposes a job with a lunch break, and "watching a programme together" presupposes a television and an evening. Contracts built from a Western middle-class repertoire will fail in a household where the husband works nights and shares a phone. The correction is simple and non-negotiable: the couple generates the items, and the therapist's role is to test them, not to supply them.

Second, the model's assumption of a dyad can be a poor fit for households with resident grandparents, polygamous arrangements, or extended family whose opinion governs the couple's decisions. A contract that ignores the mother-in-law's daily presence will be defeated by it, and the behavioural formulation must include every person whose behaviour maintains the pattern.

Third, the emphasis on direct communication can conflict with a family's preference for indirectness as a form of respect. A couple who communicate through an elder are not deficient; they are using a structure that works, and the therapist's task is to strengthen it rather than to replace it with a Western dyadic ideal.

Faith. Behavioural work is often assumed to be secular, and it need not be. Religious couples frequently have a repertoire of practices — prayer together, fasting, almsgiving, attendance — that function exactly as behavioural exchange items: small, daily, shared, meaningful. A therapist who builds the contract from the couple's devotional practice is using the model properly. The caution is that a therapist must not prescribe devotional practice as a technique; the couple chooses it, and the therapist's interest is in its relational effect.

> **DEBATE**
> "A model that requires homework, literacy and a motivated dyad is a model for the educated middle class." Argue both sides with reference to a household you know.

8.10a The two strongest critiques, and the model's answers

Critique one: it treats symptoms and calls it treatment. The model changes the ratio of positive exchanges and the couple's attributions, and it does not address the attachment injuries, the developmental history or the power arrangement that produced the distress. Gains may therefore be real and shallow, and the model's own follow-up literature shows relapse in a proportion of couples.

The model's answer. The evidence shows that behavioural change is not superficial: couples who change their exchanges and attributions report durable improvements in satisfaction at follow-up, and the mechanism is plausible — the couple's daily experience

is the relationship. IBCT's addition of acceptance addresses the deeper incompatibilities, and the model has been revised in response to this critique rather than defending its original form.

The fair verdict. The critique is strongest where the presentation is a legacy of trauma, violence or developmental injury; it is weakest where the presentation is a stable pattern of mutual disappointment. The model should be selected, not universalised.

Critique two: it is a technology of adjustment that can leave injustice untouched. A couple in which the husband does nothing in the house and the wife is exhausted can be given a contract in which she thanks him for the two things he does, and the arrangement of labour that produced her exhaustion is never examined. The model's focus on exchange can therefore make an unequal arrangement more tolerable without making it fairer.

The model's answer. The standards clarification and the fairness literature within the behavioural tradition address exactly this, and IBCT's "unified detachment" invites the couple to examine the pattern together. The model does not require the therapist to accept the couple's arrangement as given; it requires the therapist to work on behaviour that both can change.

The fair verdict. The answer is adequate in principle and frequently neglected in practice. The safeguard is procedural: the formulation must name the distribution of labour, time and money explicitly, and the therapist must ask whether the presenting complaint is a symptom of an arrangement that one partner has no power to renegotiate.

8.10b Faith, and the limits of a technique

A short section, because the model's secular reputation is misleading.

What the couple's faith might supply to the model. A repertoire of shared, repeating, meaningful behaviours (prayer, fasting, attendance, hospitality); a standard for marriage that is explicit and shared rather than private and negotiable; a community that reinforces the couple's commitments; and a vocabulary for repair (forgiveness, repentance, patience) that is richer than the model's own.

What the model might supply to the couple. Specificity about what happens on a Tuesday; a structure for raising a complaint without escalation; an acceptance of difference that is not resignation; and a plan that can be reviewed. Many religious couples have the vocabulary and not the procedure, and the procedure is what the model is good at.

The two errors. The first is to treat the couple's faith as a resource to be instrumentalised: "pray together for ten minutes each evening, it will improve your scores". The second is to treat the couple's faith as an obstacle to be worked around: "let us leave religion out of this". The first instrumentalises something the couple holds sacred; the second excludes the most important structure in their relationship.

The rule. The couple names the practices; the therapist asks what those practices do in the relationship; and where the couple wants to build on them, the model's technology — specificity, daily frequency, observability, review — is applied to something the couple

already values. A contract in which one item is "we pray together after the children are asleep, on weeknights" is a behavioural item, a devotional practice and a relationship standard in one line, and it is usually the item that survives.

8.11 Evidence note: the strongest base in the course

What exists. Behavioural couple therapy and its cognitive and integrative descendants have the largest randomised evidence base of any model in this book. Trials and meta-analyses support their effectiveness for relationship distress, with effect sizes that are substantial and durable for a majority of couples; the model is recommended in clinical guidelines in several countries. There is also good evidence for couple therapy as a treatment for depression where relationship distress is present, and for improving relationship satisfaction alongside individual symptom change.

What the evidence shows about mechanisms. Behavioural exchange, communication and problem-solving each contribute; the cognitive additions improve outcomes for couples whose distress is carried by attributions and standards; and IBCT performs comparably to traditional behavioural couple therapy with a different mechanism, which suggests that the common ingredient may be the structured, hopeful, collaborative frame rather than any single technique.

What the evidence does not show. How to select couples who will not benefit; how to prevent the deterioration seen in a minority; whether the model works as well across cultures and literacy levels as it does in the trial populations, which were largely Western and educated; and whether the model's gains survive the removal of the therapist's structure, which is the clinical question every practitioner has.

The matrix cell. "Strong randomised evidence for relationship distress and for depression treated in a couple format; mechanisms partly established; the model's limits are selection, culture and the generalisation of in-session skills to the home."

The practitioner's claim. "I use this model when the couple want a structured, time-limited piece of work and the presentation is a pattern of exchange and attribution; I measure satisfaction at intake, mid and end; and I look for the presentations the model handles badly — coercion, trauma, and a distribution of power one partner cannot renegotiate — and refer or integrate accordingly." That sentence is the model's honest advertisement.

8.12 Key terms

Term	Definition
Behavioural exchange	Reciprocal small positive behaviours, chosen by the receiver, performed daily
TBCT	Traditional behavioural couple therapy: exchange plus communication and problem-solving training
Selective attention	Noticing negatives and missing positives; a perceptual maintenance mechanism
Attribution	The explanation attached to a partner's behaviour
Relationship standard	A belief about what a marriage should be, about intimacy, fairness or loyalty

Term	Definition
Skills deficit	The skill has never been learned
Skills performance	The skill exists but is not used at home
Speaker–listener	Structured turn-taking with reflection, using a talking object
DEEP analysis	Differences, emotional sensitivities, external stress, pattern
IBCT	Integrative behavioural couple therapy: acceptance alongside change
Unified detachment	The couple examines the pattern together rather than each other
Empathic joining	Each partner's behaviour described as understandable given its history and function
Graded task	A small step toward a feared or avoided behaviour, reviewed and extended

8.13 Self-test

1. Give the five criteria for a behavioural exchange item, and test one item against them.
2. Distinguish a skills deficit from a skills performance problem, with a treatment consequence for each.
3. What are the four moves of attribution retraining?
4. Name the four columns of a DEEP analysis.
5. Why is the negative ratio important, and what does the therapist do about it?
6. What is IBCT's addition to the model, and what problem in the original did it address?
7. State the two strongest critiques and the fairest verdict on each.
8. What must a therapist ask in session one before running a communication exercise?

Answers. 1. Small, frequent/daily, cheap, observable, chosen by the receiver. Test: "be more affectionate" fails observability and receiver choice; "put your hand on my shoulder when you pass me in the kitchen" passes all five. 2. A deficit means the skill was never learned and is treated by training; a performance problem means the skill exists but is unavailable at home and is treated by examining what happens at home — usually an emotional or contextual variable — rather than by more training. 3. Name the event; name the automatic explanation; generate three alternatives; test them against what happened. 4. Differences, emotional sensitivities, external stress, pattern. 5. Because distressed couples monitor injuries, so the relationship is experienced as mostly negative even when the acts are similar in number; the therapist first teaches perception — what the partner actually did — and then adds behaviours. 6. Acceptance of unsolvable differences, through empathic joining, unified detachment and tolerance building; it addressed the finding that change strategies fail and sometimes worsen distress where the difference is an incompatibility rather than a deficit. 7. Critique one — symptom-level change, risk of relapse; fairest verdict, strongest where the presentation involves trauma, violence or developmental injury. Critique two — a technology of adjustment that leaves injustice untouched; fairest verdict, adequate in principle, neglected in practice, and the safeguard is to name the distribution of labour, time and money in the formulation. 8. Whether either partner is afraid.

8.14 Reflective task

Write your own behavioural exchange contract, as though your household were the client: three items you would give and three you would want, each filmable, daily, cheap and free of criticism. Then ask the person you live with whether the items you chose for them are the ones they would have chosen. The gap between the two lists is the technique's central lesson, and it is usually larger than students expect.

8.15 Readings for this week

- **Jacobson & Margolin (1979)**, Marital Therapy, the original behavioural manual, read for the technology.
- **Christensen & Jacobson (2000)**, Reconcilable Differences, the IBCT statement and the most useful book on this shelf for practice.
- **Epstein & Baucom (2002)**, Enhanced Cognitive-Behavioral Therapy for Couples, for standards and attributions in depth.
- **Baucom, Shoham, Mueser, Daiuto & Stickle (1998)**, the review of empirical status, for the evidence claims in 8.11.
- **Christensen, Atkins, Baucom & Yi (2010)**, the IBCT trial, for how the model is tested.
- **Gottman (1999)**, The Marriage Clinic, read critically: the observational claims are important and some of the statistics have been contested.

Read Reconcilable Differences with a couple in mind, and mark every place where the model would have to import something from Week 7. For the essay, cite the trials rather than the manuals when making outcome claims, and separate what the model has shown from what its practitioners believe. The reading-log question this week is: "what would this model have to ignore about this couple in order to work with them?" Your answer to that question is the beginning of an integrative practice.

Next: Week 9 — Emotionally focused therapy

Week 9 — Emotionally focused therapy

The question. What attachment need is underneath the anger, and how can the cycle be de-escalated so the bond can be repaired?

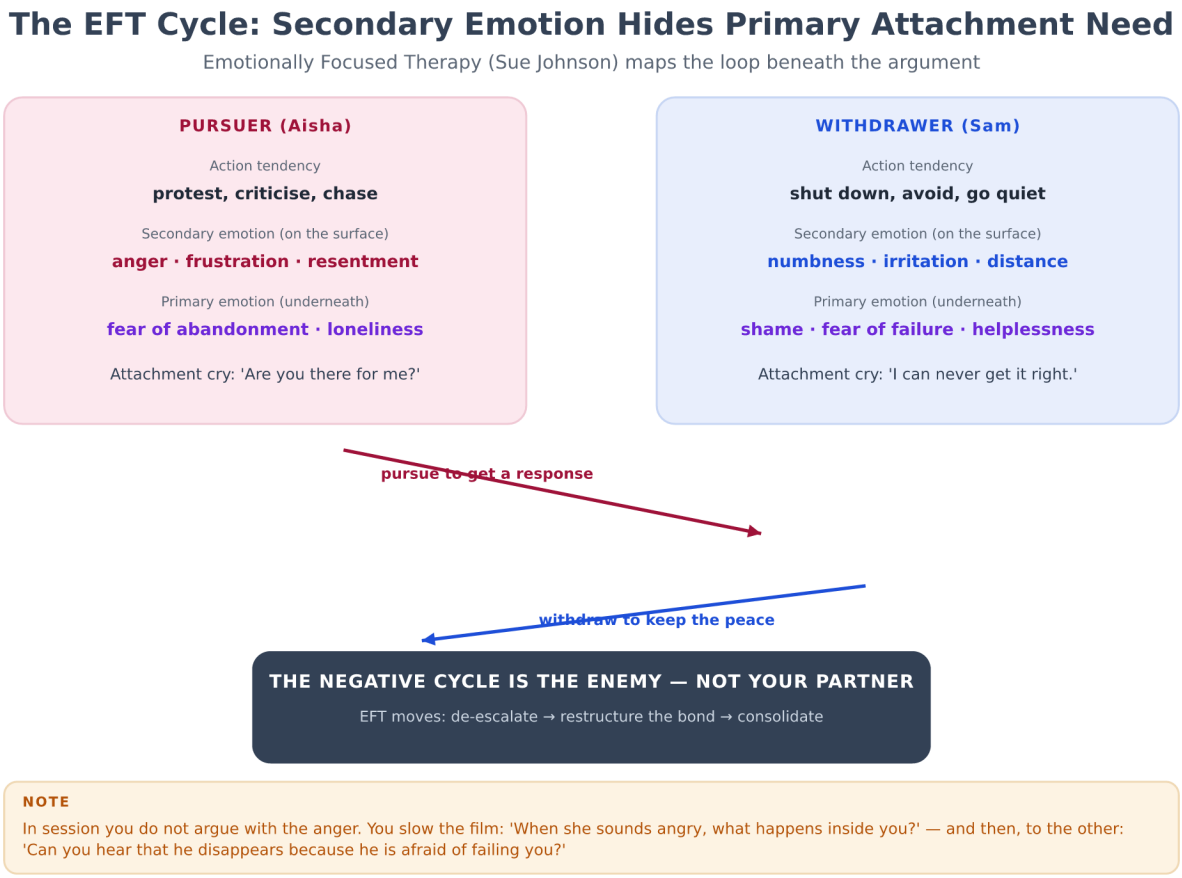


Figure 9.1 — The EFT cycle: secondary emotion and primary attachment need

Core concepts. Attachment theory as the map (see Figure 17); the negative cycle (pursue–withdraw, attack–defend, both-withdraw); primary versus secondary emotion; the "attachment cry"; softening events; the therapist as a process consultant and secure base.

Therapy in three stages. 1) **De-escalation** — identify the cycle, access primary emotion, reframe the problem as the cycle. 2) **Restructuring** — withdrawer re-engagement, pursuer softening, enactment of new responses. 3) **Consolidation** — integrate, problem-solve, prepare for recurrence.

Signature techniques.

- **Tracking the cycle:** "What happens inside you when she says that?"
- **Deepening primary emotion:** "Under the anger — is there something like fear of being alone?"
- **Enactment:** "Can you tell him that now, looking at him?"
- **Softening:** the pursuer's protest turns into an attachment expression, and the withdrawer responds.

Evidence. Among the most robust in couple therapy (over 30 trials, large effect sizes, durability in follow-up). Considered a first-line couple therapy for relational distress.

Blind spots. Requires a specific in-session emotional intensity and skills; less suited to ongoing coercive control; cultural calibration needed (the language of "needs" may feel self-indulgent to some families).

> **CASE**
> Aisha and Sam. Third session: the therapist slows a familiar argument to 20% speed. Aisha's anger is reframed as protest; Sam's silence as fear of failing. The first enactment — Aisha, quietly: "I get loud because I think you are leaving" — is the turning point. The next eight sessions are the work of making that statement safe to repeat at home.

9.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Why attachment won	The evidence, and the failure of skills training alone	Name the model EFT replaced
10–25	The map	Attachment needs, fears and strategies, drawn on one page	Map one couple from memory
25–40	Primary and secondary emotion	The anger/fear distinction, demonstrated with a volunteer	Label six feeling statements
40–60	The cycle at slow speed	A recorded session, stopped at three points	Identify each move in the EFT Tango
60–75	Enactment	The four-line grammar, rehearsed in pairs	Perform one enactment
75–85	Softening	What it is, why it is the turning point, how it fails	Write one softening sentence
85–90	Evidence and limits	Thirty trials, and the control exception	Fill the matrix row

The teaching decision that makes the week work. EFT cannot be taught by describing the cycle; it has to be heard. The lecture should play one recorded couple session of eight to ten minutes, stopped three times, and each stop should ask the class to name what the therapist did in the previous ninety seconds. The answers, collected on the board, look like a list of ordinary empathic remarks — until the third stop, when the class realises that the sequence has changed the couple's emotional state and that no interpretation, instruction or homework was involved.

The sentence the lecture must land. "The cycle is the client." Everything else in the model follows: the problem is not Aisha's anger or Sam's silence but the loop the two of them co-construct; the therapist's task is not to adjudicate, teach or advise but to slow the loop down until both partners can see it; and the change that matters is a new emotional response, offered and received, in the room.

9.2 Core concepts

Attachment theory as the map. EFT is built on Bowlby and on the adult attachment literature (MFT 703). The premises: the need for secure connection is a primary need, not a derivative of other drives; threats to the bond — real, anticipated or perceived — activate attachment behaviours that are pre-programmed and largely involuntary; and the strategies people use when the bond is threatened are the same strategies that shape their adult relationships. The map gives the therapist a way to hear a couple's hostility as attachment behaviour rather than as evidence of incompatibility.

The negative cycle. Every couple in distress is organised by a repeating sequence that has become the relationship's real subject. Three common forms: pursue–withdraw (one partner moves toward, demanding contact; the other moves away, seeking safety); attack–defend (both move toward, with criticism meeting counter-criticism); and both withdraw (both move away, and the relationship becomes a benign emptiness). The cycle is not a symptom of the couple's problems; it is their problem, and the model's reframe makes that explicit.

Primary and secondary emotion. EFT's central distinction. Secondary emotion is the feeling that appears first and is expressed: anger, irritation, coldness, contempt. Primary emotion is the softer, more vulnerable feeling underneath: fear of abandonment, shame of not being enough, grief, helplessness. The secondary emotion is a protective response to the primary, and the therapy's work is to reach the primary and to make it speakable in the partner's presence. The distinction is not moral — secondary emotion is not false — but strategic: only the primary emotion evokes care, and only care interrupts the cycle.

The attachment cry. The moment in which a partner's protest becomes an explicit statement of need and fear: "I get loud because I think you are leaving." It is the model's target event, and the therapy is designed to create the conditions for it. When it happens, the other partner's response is the therapy's second target: the withdrawer who stays, listens and answers.

The pursue–withdraw pattern, described from both sides. The pursuer's experience: "I am alone in this relationship, I have to fight to be seen, and his silence proves he does not care." The withdrawer's experience: "nothing I do is right, the criticism is relentless, and withdrawing is the only way to keep the peace." Both are describing the same loop from opposite ends, and both are accurate.

The therapist as process consultant. Not a teacher, not a referee, not a coach: a consultant on the process between the partners. The therapist tracks the cycle, names it, slows it, keeps the session safe, and offers the couple an experience of a relationship in which one person's vulnerability is met rather than punished. The therapist is also, deliberately, a secure base: steady, responsive, present, and neither shocked nor punitive.

The EFT Tango. The model's five-move micro-structure, which is what the therapist is actually doing while appearing to converse: (1) mirror the present process; (2) affect assembly and deepening; (3) choreograph an enactment; (4) process the enactment; (5) integrate and validate. Most trainees learn the five stages and then conduct sessions that

are all mirroring; the Tango is the antidote, because it makes the enactment mandatory in almost every cycle.

Softening. The turning point of stage two, and the model's signature event. The more blaming partner moves from protest to an attachment statement, delivered to the other partner, and the other partner responds from a place of care. When it happens, the couple's cycle is not merely understood but interrupted, and the change is felt in the room. Softening is not a technique to be performed; it is an event to be prepared for and then got out of the way of.

Consolidation. Stage three: the new pattern is named, practised, made robust, and connected to the couple's ordinary life; old problems are problem-solved with the new resources; and the couple is prepared for the cycle's return, which is expected rather than treated as failure.

9.2a A map of the cycle, drawn in four boxes

The single most useful artefact a trainee can produce in an EFT session, and the one that makes the model visible to the couple.

The four boxes. Top left: Aisha's secondary behaviour ("I raise my voice, I follow him"). Top right: Sam's secondary behaviour ("I go quiet, I leave the room"). Bottom right: Sam's primary experience ("I am failing her, I will never be enough, so I disappear"). Bottom left: Aisha's primary experience ("I am alone, he is leaving me, so I shout to bring him back"). Arrows run clockwise from behaviour to the other's primary experience, and back.

How the map is built in session. Not at a desk: on a whiteboard or a sheet, with the couple watching, in the therapist's own words first and then in theirs. The therapist writes the couple's language rather than clinical language — "you go blank" rather than "dissociation", "you get loud to bring him back" rather than "protest behaviour".

How the map is used. It is shown to the couple as an external object: "this is the thing we are fighting, not each other." That externalisation is the model's reframe made concrete. The couple is then asked to correct the map — a correction is the first collaborative act of the therapy — and the corrected version is photographed or copied for them.

The two questions that fill the bottom boxes. "When you get loud, what is happening under the loud?" and "when you go quiet, what is happening under the quiet?" Trainees should notice how hard these questions are to ask without sounding like an accusation, and how quickly the couple answer when the tone is right.

What the map does not contain. No history, no diagnosis, no third parties, no solution. The map's purpose is to make the cycle the client, and any addition dilutes that purpose. History comes later, or not at all; solutions come in stage two and three, when the map is already shared.

A diagnostic use of the map. If the couple cannot agree on the map after several attempts, or if one partner's behaviour cannot be described as protective of anything, the therapist should consider whether the presentation is not a cycle at all: coercive control,

active addiction, or a psychiatric condition will each produce a pattern that looks like a cycle and is not one (see 9.7). That possibility is the model's most important screening question, and it belongs in the first two sessions.

9.2b Attachment strategies, and how they present in a couple session

The model's assessment depends on recognising each partner's strategy in the first twenty minutes, before any history is taken. Five presentations, each with what the therapist sees and what is underneath.

Hyperactivating (anxious) strategy. What you see: fast speech, monitoring, repeated questions, protest when the partner is late, escalating until contact is achieved, difficulty ending a conversation. What is underneath: a working model in which the other person's availability is uncertain and must be continuously tested. The therapist's move: slow the pace, name the protest as protest, and reach the fear underneath without dismissing the strategy as excessive.

Deactivating (avoidant) strategy. What you see: brevity, gaze away, a focus on tasks and facts, early withdrawal from conflict, discomfort with the partner's tears, a tendency to be helpful rather than present. What is underneath: a working model in which need is dangerous and self-sufficiency is safety. The therapist's move: narrow the question, accept small answers, and treat a single vulnerable sentence as a major achievement.

Disorganised strategy. What you see: approach and withdrawal in the same minute; intense pursuit followed by abrupt coldness; contradictory accounts of the same event. What is underneath: a history in which the source of comfort was also the source of fear. The therapist's move: predictability above all — the same time, the same frame, the same tone — and a slower pace, with an eye on trauma work (MFT 719).

The controlling-caregiving strategy. What you see: one partner managing the other's life, organising, advising, and experiencing the other's independence as rejection. What is underneath: safety achieved by being needed. The therapist's move: reframe the care as the strategy it is, and ask what would happen if the other partner managed something alone.

The controlling-punitive strategy. What you see: monitoring, rules, consequences, humiliation, and a partner who has become cautious. What is underneath: often fear of abandonment combined with a need for control. The therapist's move: **this is not a cycle position.** It is a safety matter, and the model's attachment language must not be used to soften it into a mutual pattern (see 9.7's screening question and MFT 719).

Why this table earns its place. Trainees who can name a strategy in the first ten minutes can predict the cycle, choose the order of work, and avoid the common error of asking a deactivating partner to do the emotional labour of an activating one's preferred conversation. The table is also the model's ethical instrument: it makes the punitive strategy visible as something other than a variant of avoidance.

9.3 The therapy: three stages, and what happens in each

Stage one — de-escalation (sessions 1–8, approximately). The cycle is identified, made explicit, and named as the couple's common enemy. Both partners' primary emotion is accessed and articulated, at first in individual conversation with the therapist and later in each other's presence. The reframe is delivered: the problem is not that she is angry and he is cold, but that both are afraid of losing the other and neither can say so. The stage ends when the couple can describe the cycle together, without blaming, and when both can name their own primary experience.

What the therapist does in stage one. Takes an attachment history that is also an assessment (how did each partner learn to get comfort? who was available? what happened when they were frightened?); tracks the cycle in the session rather than taking a report of it; slows the interaction to a fraction of its normal speed; marks each partner's secondary behaviour as it occurs; asks the deepening question and waits; and reframes the couple's problem as the cycle, repeatedly, in slightly different words.

Stage two — restructuring (sessions 9–16, approximately). The withdrawer is helped to re-engage: to state their own fear and their own withdrawal, to risk staying in the conversation, and to answer the partner's attachment cry. The pursuer is helped to soften: to move from protest to an explicit attachment statement, and to hear the withdrawer's answer without reverting to accusation. New cycles are enacted in the room and then tried at home. The stage ends with a softening event — the one the model considers the predictor of durable change.

What the therapist does in stage two. Choreographs enactments (always with the same grammar: notice, invite, hold, process); blocks the old pattern in the moment ("wait — that is the old move, can you say what was underneath it?"); keeps the emotion at a workable intensity; and refuses to let the couple resolve the moment with an argument.

Stage three — consolidation (sessions 17–20, approximately). The new cycle is named and made describable; the couple's old problems are revisited with the new pattern available; the return of the cycle is anticipated and planned for; and the therapy ends with an explicit account of what the couple now knows about themselves. EFT's typical course is eight to twenty sessions for a couple without significant trauma, and longer where trauma is present.

What the therapist does in stage three. Becomes less active; hands the process to the couple; tests the new pattern under stress (a real, current disagreement); and closes with a review session at three and six months.

The four P's of the EFT assessment. Pattern (the cycle), Primary emotion (each partner's vulnerability), Pursuer/withdrawer positions (who does which, and whether it is stable or switches), and Predisposing attachment history (what each partner learned). The formulation is written in one page, in the couple's language, and shared with them in session two or three.

9.3a Stage two in slow motion: the withdrawer's re-engagement

Stage one gets the attention in teaching; stage two is where the model's skill lives. A worked sequence with the Ogundipes (mother 44, father 47), ten minutes of a session, with the therapist's moves named.

The starting point. The couple have named the cycle. She pursues with complaints about his evenings; he withdraws into work and his phone. Both can describe the loop. Neither has yet said anything vulnerable.

Move one — the invitation to the withdrawer. "I want to try something different, and it may feel strange. I am going to ask you to say something to her that you have been keeping in your silence. If it is too much, we stop." The sentence gives the withdrawer control, which is essential: a withdrawer who is pushed will withdraw further.

Move two — the concrete question. Not "how do you feel?" but "when she says 'you are never here', what happens in you in the first three seconds?" The narrow time frame and the request for a sequence make the question answerable by a man who has spent twenty years not answering feeling questions.

His answer. "I go tight. And then I think: here we go, I cannot win, so I say nothing." The answer is a sequence, not a feeling, and it is enough.

Move three — the deepening. "And when you think 'I cannot win', what is that like for you about her?" The question moves from his internal state to his experience of the relationship, which is where attachment material lives. His answer: "that I am a disappointment to her." The therapist says the word back: "a disappointment." Silence.

Move four — the enactment. "Can you say that to her? Not about her. To her: 'I go quiet because I think I am a disappointment to you.'" He does, badly, looking at the floor. The therapist does not correct the delivery; the content is the achievement.

Move five — processing the enactment with the pursuer. "What happened in you when he said that?" Her answer: "I did not know that. I thought he did not care." The therapist reflects the new information and does not add an interpretation.

Move six — blocking the old move. She then says: "but he still does not help with the children." The therapist blocks, gently: "that is true, and it is a different conversation. Right now, can you stay with what he just said?" The block is the model's most important discipline: the old pattern will attempt to reclaim the session within thirty seconds of any vulnerable moment, and the therapist must hold the space.

Move seven — the pursuer's first softening step. "Can you tell him what it was like to hear it?" She says: "it is a relief, and it makes me frightened, because I have been fighting someone who was already beaten." That sentence is not yet a softening event, and it is its necessary precursor: the pursuer's protest has begun to be replaced by an accurate picture of the other.

The sequence's logic. Re-engagement before softening, and both before any problem-solving. A therapist who reverses the order produces a couple who understand each other and continue to fight.

9.4 Techniques, with the grammar of each

Tracking and reflecting the cycle. The therapist narrates the process as it happens, in the couple's language, without blame: "so you reach for him, and he goes still, and the stiller he goes the louder you get." The narration is the intervention; it converts a fight into an object that both can see.

The deepening question. Four forms: "what happens in you when she says that?" (sequence); "under the anger, what is there?" (primary emotion); "what is it like to be the one who is always the one who is blamed?" (position); "what do you do with that feeling?" (strategy). The question is asked slowly, once, and followed by silence.

The evocative response. The therapist does not paraphrase the feeling; the therapist responds to it, in the first person, briefly: "that sounds unbearably lonely." The response is what deepens the affect, and it is also what makes the therapist a secure base. It is not a technique for producing tears, and a therapist who uses it to that end is doing something else.

Enactment. The four-line grammar: notice ("I see your jaw, and I think something just happened"); invite ("can you tell her, rather than me?"); hold ("stay there a moment, look at her"); process ("what was it like to say that, and what was it like to hear it?").

Catching the bullet. A specific EFT move for the moment a partner attacks: the therapist intercepts the criticism, translates it into the need underneath, and offers it back as an attachment statement. "So when you say he never listens, what I hear underneath is 'I am frightened that I am not important to him'. Is that close?" The move protects the other partner from the attack and gives the attacker a truer sentence.

Reframing the cycle as the enemy. The reframe is repeated in every session in slightly different words, and the couple is invited to name the cycle themselves: "what would you call this thing that happens between you?" Couples produce names — "the spiral", "the cold war", "the dance" — and the name is used from then on.

Softening. Not a technique but an event, prepared by stage one and choreographed in stage two: the pursuer's protest converts into an attachment statement delivered with eye contact, the withdrawer stays present, and the therapist holds the moment and then gets out of it. The failure modes are instructive (see 9.7a).

The attachment history interview. Five questions, asked of each partner: who did you go to when you were frightened as a child? What did they do? What happened when you were upset and nobody came? What did you learn to do with your own need? What do you still do? The answers produce the couple's predispositions, and they are the most reliable predictor of which partner will pursue and which will withdraw.

9.4a A deepening sequence, scripted in eight turns

The technique most often taught badly, because it looks like ordinary conversation. Eight therapist turns on one content, with the deepening marked.

Content: a husband has said that his wife "is never satisfied".

1. **Sequence.** "What happens in you when she says 'you never do enough'?" — not "how do you feel?"
2. **Specificity.** "Where do you notice it — in your chest, your jaw, your hands?"
3. **Word.** "What would you call that feeling, in your own words?"
4. **Deepening.** "And under that — is there anything else?"
5. **Evocative response.** "That sounds exhausting. To be working that hard and to hear that you are not enough."
6. **Position.** "What is it like to be the one in this marriage who is always slightly failing?"
7. **Attachment frame.** "When you hear that you are failing, what do you expect will happen next?"
8. **Enactment invitation.** "Can you say that to her — the part about expecting her to leave?"

What makes it a sequence and not a list. Each turn follows from the previous answer, and the therapist never moves on until the answer has been received, repeated in the client's own word, and allowed to sit. The four commonest errors are: jumping from turn one to turn seven (interpretation); supplying the word at turn three (the client says "I suppose it is shame?" and the therapist answers it for him); skipping the evocative response, which is what makes the deepening tolerable; and asking turn eight before turn six has been answered, which produces a performance rather than a disclosure.

The exercise. In pairs, run the eight turns on a mild content, and mark each turn as the therapist's question or the therapist's response. A therapist who produces eight questions and no responses is conducting an interview, not a deepening.

9.5 The first session with Aisha and Sam

The week's case, in the model's own sequence.

The presentation. Aisha (33) and Sam (36), married six years, two children. Presenting complaint: "we argue every night about nothing and then he disappears into his phone." Aisha is loud, fast, tearful. Sam is quiet, polite, and answers in single sentences.

Minutes 0–10. The therapist greets both, states the frame, and asks one question that is not about the problem: "what did you two do together before the children?" Aisha laughs; Sam answers with a specific memory. The question is not small talk: it establishes the bond as the subject, and it produces a moment of shared history in a session that will otherwise be about conflict.

Minutes 10–25. Each partner describes the problem. The therapist does not intervene in the content, and watches the process: Aisha's speed, Sam's stillness, the point at which Sam's eyes drop (when Aisha says "he does not care"), the point at which Aisha's voice rises (when Sam says nothing).

Minutes 25–35. The first tracking move. "Can I stop you both for a moment and say what I am seeing? You [Aisha] get faster as he gets quieter, and the faster you get the quieter he goes. It looks like a loop that neither of you designed. Is that what it feels like from

inside?" Both say yes. Sam adds: "I just want it to stop." The therapist marks the word: "stop."

Minutes 35–45. The deepening, addressed first to Sam, because the withdrawer is the harder case and the session's structure should not always follow the louder partner. "When she gets faster, what happens in you in the first few seconds?" Sam: "I go blank." Therapist: "and the blankness — what is it protecting?" Sam, after a pause: "I think — that I am not enough for her." Aisha's face changes.

Minutes 45–50. The therapist does not pursue the enactment yet. Instead, she names what happened: "in this room, in the last five minutes, something different has been said." She asks Aisha what it was like to hear it, and Aisha says, "I did not know he thought that", and then, to Sam, "I am not angry because you are not enough. I am angry because I am alone." The first attachment statement has arrived in session one, and the therapist's task is to hold it, not to build on it.

Minutes 50–55. The frame: eight to twenty sessions, weekly, both partners present; a task that is not behavioural but observational ("this week, notice the moment the loop starts, and write down what happened just before"); and the formulation offered in one sentence: "the problem is not that you fight; it is that when you fight, both of you end up more alone, and the loop is what we are going to work on."

What the therapist did not do. Did not take a full history; did not ask about the children's needs; did not adjudicate a single dispute; did not give advice; and did not attempt an enactment in session one beyond the moment that arose naturally.

9.6 Case: the Adeyemis, read through attachment

The recurring family, this time through the model that best explains the father's absence.

The presentation. Amara (14) has refused school for five weeks; her father Tunde is in Lagos; her mother Funke has taken leave; her brother Segun (11) is silent.

The attachment reading. Funke's attachment strategy is hyperactivation: she monitors, pursues, and escalates until contact is achieved, because in her history distance preceded loss. Tunde's strategy is deactivation: he minimises need, works, provides, and stays away, because in his history need was punished. Amara is living inside the collision: her mother's intensity is experienced as surveillance, her father's absence as confirmation that she is not worth staying for, and her symptom is a third strategy — she refuses the world.

The cycle in this family. Funke pursues Amara; Amara withdraws; Funke pursues harder and telephones Tunde; Tunde instructs and disengages; Funke's pursuit intensifies. The parents' marital cycle is enacted through the child, which is the model's account of how a child's symptom can carry an adult's attachment dilemma.

What EFT would do, and in what order. First, name the cycle with the parents, without the children present: "when you [Funke] feel alone, you move toward; when you [Tunde] feel criticised, you move away; and the more each of you does what comes naturally, the more alone you both are." Second, access each parent's primary emotion: Funke's terror

of being left to manage alone, Tunde's shame at failing as a provider and a father. Third, an enactment between them, with the children absent, in which each says the primary thing to the other. Fourth, bring the children back into a session where the parents speak from the new position — which is the model's mechanism for the child's symptom, since a child who sees her parents reach each other no longer has to hold the family's anxiety in her refusal.

What the model cannot do here without adaptation. The father is physically absent, and enactment down a telephone line is a poor instrument: the couple need face-to-face sessions. EFT's structure assumes both partners in the room, and a family whose work pattern separates them needs either a block of sessions when the father is home, video sessions with attention to the medium, or a decision about whether the couple work can begin at all. This is a practical problem the model's literature rarely addresses and which half the world's families present.

The formulation in one sentence. "This is a family in which both parents are frightened of the same thing — being left to carry it alone — and they have each developed a strategy that makes the other's fear worse, while their daughter has found a way of holding the family's attention that costs her her adolescence." That sentence is EFT's contribution to the case, and it is compatible with every formulation in Weeks 2 to 8.

9.7 Annotated transcript: the softening event

Fourteen turns from the eleventh session with Aisha and Sam — the model's turning point, in the room.

> **T1 Therapist:** We have come a long way. Do you want to try the thing we have been preparing for?
> **T2 Aisha:** (nods) I am frightened I will cry.
> **T3 Therapist:** If you cry, I will stay here, and so will he. Sam, are you able to stay if she cries?
> **T4 Sam:** (quietly) Yes.
> **T5 Therapist:** Aisha, what do you want him to know about the nights when you are loud?
> **T6 Aisha:** (long pause) That I am not angry. That I am terrified. I am terrified that one day you will decide that this is too much and you will go, and I will have been right all along.
> **T7 Therapist:** Say the last part to him.
> **T8 Aisha:** (to Sam) I will have been right all along. About not being the kind of person people stay for.
> **T9 Therapist:** (pause) Sam. What is happening in you?

> **T10 Sam:** (long silence) I want to go to her. I always wanted to go to her.
> **T11 Therapist:** Tell her that.
> **T12 Sam:** (to Aisha) I am not going anywhere. I never was. I go quiet because I think I am already failing you, and I do not know how to fail in front of you.
> **T13 Therapist:** (pause) Stay with each other for a moment. Nobody needs to say anything.
> **T14 Therapist:** (after a minute) What just happened between you?

Annotations. T1 names the task, which is the model's transparency: the couple know what they are attempting, and the therapist does not spring it on them.

T2–T4 is the safety architecture. Her fear of crying is named, the therapist commits to staying, and the other partner is asked explicitly whether he can stay. That question is doing three things: it prepares the withdrawer for his role, it makes his staying an act rather than an accident, and it protects the session if he cannot.

T5 is the invitation, framed as information for the other partner rather than as an exercise in expression. "What do you want him to know" is a more workable question than "how do you feel", because it gives the emotion a destination.

T6 is the attachment cry in its classic form: the protest ("I am not angry") giving way to fear ("I am terrified"), and then to the deepest layer ("I will have been right all along"), which is the sentence the therapy has been working toward for eleven sessions.

T7 is the smallest and most important instruction in the transcript. The therapist does not comment on what a good thing has just been said; she asks for it to be said to the person it is about. That single move converts a disclosure to the therapist into contact between the partners.

T9–T12 is the withdrawer's response, and the order of the therapist's moves matters: she asks what is happening before asking him to act, so that his action comes from his own experience rather than from compliance. His sentence ("I do not know how to fail in front of you") is the male withdrawer's primary emotion in its most usable form: shame.

T13 is the hold. A therapist who speaks here — to summarise, to praise, to interpret — destroys the event. One minute of silence in the presence of two people meeting each other is the whole of the technique.

T14 is the process question, asked afterwards, and it is what makes the event available to the couple as knowledge rather than as a moment that happened to them.

What the trainee should notice. The therapist's total contribution is about forty words. The model's difficulty is not in knowing what to say; it is in knowing when to say almost nothing, and in having built the structure that makes the nothing safe.

9.7a Three softenings that failed, and why

Failure one — the manufactured softening. A trainee, anxious to reach the turning point, asked a pursuer in session four: "can you tell him you are afraid he will leave?" The client said the sentence, the withdrawer looked at the floor, and nothing happened. The cause: stage one was incomplete. The couple had not yet experienced the cycle as their common enemy, and the words were the therapist's, not the client's.

Failure two — the softening that became an indictment. A pursuer's attachment statement was delivered with an edge: "I am afraid you will leave, like everybody else in my life." The withdrawer heard an accusation and defended. The cause: the statement contained a second sentence aimed at him. The therapist's job at that moment is to separate the two sentences and to work with the first alone.

Failure three — the softening with no response. The pursuer softened beautifully, the withdrawer said nothing at all, and the therapist filled the silence with an interpretation. The cause: the withdrawer had not been prepared for stage two — no re-engagement work, no rehearsal of his own fear — and the therapist's interpretation relieved her own anxiety at the cost of the couple's moment. The correction: a rehearsal session with the withdrawer alone, in which his fear is put into words with the therapist before it is asked for in the room.

The general lesson. Softening cannot be produced; it can be prepared. Every failed softening in the recordings is a stage-two move attempted before stage one was complete, or a therapist's need met at the couple's expense.

9.8 Laboratory: deepening and enactment

Sixty minutes, triads, two skills only.

Skill one — the deepening question (twenty-five minutes). One student speaks about a mild current frustration with a person they know; the therapist practises the four deepening forms in sequence, with at least ten seconds of silence after each; the observer counts the seconds and notes whether the therapist paraphrased, interpreted or responded. The target: three consecutive deepening questions with no interpretation and no advice.

Skill two — enactment with processing (thirty-five minutes). The same triad, with a different content. The therapist notices a moment, invites the speaker to address the other person (played by the observer), holds for ten seconds, and then processes both sides. The observer checks the four-line grammar and records the processing questions verbatim.

Marking rubric.

Criterion	Weight	Full marks
Deepening	30%	Four forms used; sequence questions before feeling questions; silence held
Evocative response	15%	Brief, first-person, responsive rather than interpretive
Enactment grammar	30%	Notice, invite, hold, process — all four present and in order
Blocking the old move	15%	At least one attempt to reclaim the session is blocked kindly and firmly
No rescue	10%	No filling of silence, no advice, no premature summarising

The debrief question. Which was harder: asking the deepening question, or saying nothing afterwards? The answer is always the second, and the seminar's task is to make the second tolerable.

9.9 Six errors in EFT practice

1. **Staying in stage one.** Months of beautiful cycle-tracking with no enactment and no softening. Fix: a written stage plan, and a review at session eight asking whether both partners can name their own primary emotion.

2. **Chasing affect.** Pushing for tears to demonstrate that the model is working. Fix: the evocative response rather than the demand, and a supervision question — whose need is being met?
3. **The interpretation in place of the question.** "You are afraid of abandonment" delivered as a formulation. Fix: the deepening question, and the client's word rather than the therapist's.
4. **Ignoring the body.** Long verbal sessions with no attention to breath, posture and gaze. Fix: the noticing move, and slowing down physically as well as verbally.
5. **Missing coercion.** Running the model with a couple in which one partner is afraid, and treating the fear as a cycle position. Fix: the screen, asked of each partner alone, and a different treatment pathway (9.7's screening question, and MFT 719).
6. **Softening as a performance.** A therapist who needs the turning point produces it. Fix: stage discipline, and a supervisor who asks what the therapist wanted from the session.

9.9a The couple who arrive at stage two too early

A common clinical situation and its handling. A couple arrives for session three having had an intense conversation at home after session two: he cried, she held him, and both now believe the therapy is nearly finished. The therapist's temptation is to accept the story and move to consolidation.

Why not. The event happened without the cycle having been named, without either partner's position having been understood, and without the therapist present to slow it down. It is a genuine moment and it is fragile: the first return of the old pattern will be experienced as a betrayal of a promise, and the couple will conclude that the change was illusory.

What to do instead. Honour the event fully — ask for the details, name what each partner did, and let the couple describe it. Then place it in the map: "so the two of you found a way to do at home what we have been practising here. What I want to know is what will happen when the loop comes back, because it will." The answer prepares the couple for recurrence, which is the difference between a good week and a change.

The general rule. In EFT the sequence protects the change. A softening that happens off-sequence is not to be dismissed, and it is not to be mistaken for the work: it is evidence that the couple can do it, and the therapy's task is to make it repeatable.

9.10 Culture, faith and the model's critique

Where the model travels well. Attachment is not a Western invention; the need to know that one's belonging is secure is recognisable in every family system this book has described. The model's focus on process, its refusal to adjudicate content, and its respect for the couple's own language make it adaptable. Its central insight — that anger is often protest and silence is often fear — is useful in any culture, including those in which anger is more available than fear in men and silence more available than protest in women.

Where it strains. Three places, all documented in the model's own literature.

The language of needs can sound self-indulgent. Where a couple's moral vocabulary is duty, sacrifice and endurance, a therapist who asks about "your needs" may be heard as inviting selfishness. The adjustment is to translate: "what do you owe each other, and what happens to you when you cannot give it?" — the same structure in the family's own moral language.

The expression of vulnerability is gendered and cultural. Many men in many cultures have no permissible vocabulary for fear and shame in front of a partner, and a model that requires it may produce a session in which the man is silently failing. The adjustment is the one the model's own theorists recommend: find the partner's own word for the experience, which may be "respect", "being a man", "failing my father" or "shame", and use it.

The couple is assumed to be the decision-making unit. Where extended family, religious authority or economic dependence determines the couple's options, the model's work with the dyad can produce a new emotional understanding that the household cannot accommodate. The adjustment is to include the relevant third parties in the formulation and, where possible, in a session.

Faith. EFT's attachment vocabulary maps onto religious vocabulary with unusual ease: being known, being held, being faithful, being forgiven. A couple whose faith frames the marriage as a covenant will often find the model's language of "safe haven" and "secure base" congenial, and the therapist can use their vocabulary without appropriating it. The caution is the model's own: the therapist is not a pastor, the session is not a confession, and the spiritual frame belongs to the couple.

> **DEBATE**
> "EFT is the best-evidenced couple therapy and therefore should be the default. Discuss what that claim leaves out."

9.10a The two strongest critiques, and the model's answers

Critique one: the model's gender politics are unexamined. In the classic pursue–withdraw formulation, the woman pursues and the man withdraws, and the therapy's task is to soften her protest and to re-engage him. Feminist critics have observed that this can pathologise the partner who is protesting an unequal arrangement and reward the partner who benefits from it: her anger becomes a "secondary emotion", his silence becomes "fear", and the solution is her vulnerability plus his presence, not a redistribution of labour or power.

The model's answer. The formulation is descriptive, not prescriptive; the positions are not gendered in principle and switch frequently; and the model's later writing addresses power and context explicitly. The withdrawer's re-engagement is not a reward but a demand — he is asked to become available, which is a change in the arrangement, not a defence of it.

The fair verdict. The critique has force where the presenting problem is an unequal division of labour and the therapy never names it. The safeguard is procedural: the EFT

formulation must include the material arrangement — who does what, who earns, who decides, who can leave — and the therapist must ask whether the cycle is a dance between equals or the symptom of an arrangement one partner cannot renegotiate.

Critique two: the model requires an intensity many settings cannot provide. Weekly fifty-minute sessions with both partners, a therapist trained to a high standard, and a couple able to attend together for four to five months. In a public clinic with a waiting list, or a family in which one partner works away, the model's conditions are unavailable, and what gets delivered is a diluted version whose evidence does not transfer.

The model's answer. EFT has been adapted for shorter formats, for groups, for individuals, and for telehealth; the trials include brief versions; and the model's stage structure can be used diagnostically even where full treatment is not possible.

The fair verdict. The answer is partly satisfactory. The evidence supports the full model, and the adaptations are promising rather than established. A therapist delivering a six-session version in a busy clinic should describe it as an adaptation and measure it, rather than claiming EFT's trial results.

9.11 Evidence note: the strongest couple evidence in the course

What exists. EFT has a substantial randomised evidence base — over thirty trials at the time of writing, including several from independent teams — with large effect sizes for relationship distress, high rates of recovery, and durability at two-year follow-up. It performs well against behavioural couple therapy in head-to-head trials and is recommended as a first-line couple therapy in several clinical guidelines. There is also evidence for EFT with couples where one partner has depression, with trauma survivors, and in family-format adaptations.

What the evidence shows about mechanism. Process research supports the model's central claims: the depth of in-session emotional processing predicts outcome; softening events predict durable change; and the therapeutic alliance and the therapist's ability to regulate the session's intensity are associated with outcome. This is unusual in the field — the model has both outcome and process evidence for the same mechanism.

What the evidence does not show. How the model performs where violence or coercive control is present (it is generally excluded from trials, and should be); how it performs across cultures, since most trials are Western; how much training is required for the effect (a serious implementation question); and whether the model's benefits persist beyond two years, which is the limit of most follow-up.

The matrix cell. "Strong randomised evidence with large effect sizes and durability; process evidence for the softening event as the mechanism; first-line status in guidelines; exclusion criteria include ongoing violence, which is also the model's principal clinical limit."

The practitioner's claim. "EFT is the couple model I would choose first for relational distress where there is no coercion, because it has the best outcome and mechanism evidence in the field; I screen for violence before starting; I use the stage structure as a

map and a discipline; and I measure satisfaction and alliance at every fourth session." That sentence can be defended in a viva without overstatement.

9.12 Key terms

Term	Definition
Negative cycle	The repeating sequence of pursue–withdraw, attack–defend or both-withdraw that organises the couple's distress
Secondary emotion	The expressed, protective feeling: anger, contempt, coldness
Primary emotion	The vulnerable feeling underneath: fear of abandonment, shame, grief, helplessness
Attachment cry	Protest converted into an explicit statement of need and fear
Pursuer	The partner who moves toward, demanding contact
Withdrawer	The partner who moves away, seeking safety
EFT Tango	Five-move micro-structure: mirror, deepen, choreograph, process, integrate
Enactment	Turning a disclosure to the therapist into contact between the partners
Softening	The pursuer's protest becoming an attachment statement, met by the withdrawer's response
Catching the bullet	Intercepting a criticism and translating it into the need beneath
De-escalation	Stage one: the cycle identified, primary emotion accessed, the problem reframed
Restructuring	Stage two: re-engagement, softening, new cycles enacted
Consolidation	Stage three: integration, problem-solving, preparation for recurrence

9.13 Self-test

1. Distinguish primary and secondary emotion, and explain why the distinction is strategic rather than moral.
2. Give the three common forms of the negative cycle.
3. What is the EFT Tango, and why does it matter in training?
4. What is a softening event, and what predicts it?
5. Why is the withdrawer usually worked with first in stage two?
6. State the model's central evidence claim and its principal exclusion.
7. What are the two strongest critiques and the fairest verdict on each?
8. What is the screening question every EFT therapist must ask, and of whom?

Answers. 1. Secondary emotion is expressed and protective (anger, contempt); primary is the vulnerable feeling beneath (fear, shame, grief). The distinction is strategic because only primary emotion evokes care and interrupts the cycle; the secondary feeling is no less real. 2. Pursue–withdraw, attack–defend, both-withdraw. 3. A five-move micro-structure — mirror the process, deepen the affect, choreograph an enactment, process it, integrate and validate — which matters because trainees otherwise mirror for

the whole session and never enact. 4. The pursuer's protest becomes an explicit attachment statement delivered to the partner, who responds from care; it is predicted by completed de-escalation and by the withdrawer's prior re-engagement. 5. Because the withdrawer is the harder and more defended position, and because the pursuer's softening requires a partner who can stay; re-engagement before softening. 6. Over thirty trials, large effect sizes, durability, first-line status; the principal exclusion is ongoing violence or coercive control.

1. Critique one — unexamined gender politics and the risk of pathologising protest against an unequal

arrangement; fairest verdict, it has force where the material arrangement is never named. Critique two — the model requires intensity and resources many settings lack; fairest verdict, adaptations are promising not established, and should be described as adaptations. 8. Whether either partner is afraid — asked of each partner individually, in the first or second session, and recorded.

9.14 Reflective task

Write down the last time you were angry with someone you love. Then write, in three sentences, what was underneath the anger: what you were afraid of, what you wanted, and what you did instead of saying it. Then decide whether you could have said the three sentences to that person at the time, and what would have happened. Bring it to the seminar; the exercise is the model applied to the therapist's own attachment strategy, and it is the most reliable way to understand why softening is difficult.

9.15 Readings for this week

- **Johnson (2004)**, The Practice of Emotionally Focused Couple Therapy, the manual, read chapter by chapter alongside the stage plan.
- **Johnson & Greenberg (1985)**, the original trial, for how the model was first tested.
- **Greenberg & Johnson (1988)**, Emotionally Focused Therapy for Couples, the earlier statement, useful for the emotion theory.
- **Johnson (2008)**, Hold Me Tight, the popular version, read to see how the model explains itself to couples — and to see what it simplifies.
- **Burgess Moser et al. (2015)**, on the mechanism of change, for the process evidence.
- **Wiebe & Johnson (2016)**, the meta-analysis, for the outcome claims.

Read the manual with a transcript in hand and mark every therapist turn as mirroring, deepening, choreographing, processing or integrating. Twenty minutes of that exercise will do more for your fidelity than the whole of the manual's theory chapters. For the essay, separate the model's outcome evidence from its process evidence and say what each establishes. The reading-log question is: "what does this model require the therapist to feel, and what happens if the therapist cannot?"

Next: Week 10 — Solution-focused brief therapy

Week 10 — Solution-focused brief therapy

The question. What is already working, and what would one small step look like?



Figure 10.1 — Solution-focused brief therapy in five moves

Core concepts. Solution talk versus problem talk; the miracle question; scaling; exceptions; coping questions; the client as expert; small, observable change; "if it works, do more of it".

Therapy. Conversational, non-normative, often 3–6 sessions. The therapist listens for competence and deliberately amplifies it; homework is usually "notice what works".

Signature techniques.

- **Miracle question:** "Suppose tonight a miracle happens and the problem is solved. What will you notice first tomorrow morning? Who else will notice?"
- **Scaling:** "On 0–10, where are you? What would one point higher look like this week?"
- **Exception questions:** "Tell me about a time the arguing stayed small. What were you doing differently?"
- **Coping question:** "With everything you have faced, how have you managed to keep going?"

Evidence. Good evidence for depression, with brief formats showing effects comparable to longer treatments in some trials; widely used in child protection and school settings.

Blind spots. Can bypass necessary grief or trauma work; risks ignoring risk; may feel invalidating when a family needs to tell the story of what happened.

> **PRACTICAL**
> Full miracle-question sequence in pairs (8 minutes), then a one-point scaling and a concrete task. Observer records: did the therapist ask for concrete behavioural detail, and did the client's posture change?

10.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The wager	Why a therapy with no theory of pathology might still work	Name what solution-focused work refuses to know
10–20	Problem talk and solution talk	Two transcripts of the same family, ten minutes apart	Mark every problem-focused sentence
20–35	The miracle question	The full sequence, demonstrated live with a volunteer	Write the six follow-up questions
35–50	Scaling and exceptions	One number, one exception, one small step	Run a scale with a partner and find an exception
50–60	The session structure	The five-move spine, used in a real first session	Map a session they have observed
60–72	The end-of-session task	Compliments and the "notice what works" task	Compose a task for a case
72–82	Risk and grief	When the model must stop asking about solutions	Design a safety override
82–90	Evidence and critique	The trials, the transfer problem, the ethical objection	Fill the matrix row

The teaching decision that makes the week work. Solution-focused work is the easiest model in the book to caricature and the hardest to perform without sentimentality. The lecture should therefore begin with a critique rather than an endorsement: two transcripts of the same first session, one conducted by a therapist who asks about problems and one by a solution-focused therapist, and the class is asked which family is more likely to return — and then which family's safety has been assessed. The answer is uncomfortable in both directions, and that discomfort is the week's engine.

The sentence the lecture must land. "The model is not a refusal to hear pain; it is a decision about where to put the therapist's attention." A student who can say that sentence and then demonstrate the miracle question without irony has understood the week.

10.2 Core concepts

Solution talk versus problem talk. The model's founding distinction. Problem talk describes what is wrong, why, since when, and who is responsible; solution talk describes what would be different, what is already working, and what the next small step is. Both are forms of conversation, and they have different effects: problem talk organises attention around injury and blame, solution talk organises it around competence and possibility. The therapist's craft is to move the conversation from the first to the second without denying the first.

Why the distinction is not merely optimistic. The model's claim is not that problems are unreal; it is that the description of a problem does not contain the description of its solution, and that time spent elaborating the former does not automatically produce the latter. A family that has told its story of failure for two years has an extremely detailed account of what does not work and almost no account of what does. The therapist's questions build the missing account.

The client as expert. The model's epistemological position, taken further than any other in this course. The family knows its life; the therapist knows only how to ask questions that make the family's knowledge usable. This has a practical consequence: the therapist does not supply goals, does not decide what counts as improvement, and does not interpret the family's account. It also has an ethical consequence: the model is unusually respectful of a family's own values, including values the therapist does not share.

Exceptions. The model's central observational claim: the problem is never present 100% of the time, and the times when it is absent or smaller contain the ingredients of the solution. An exception is not a compliment; it is data. "Tell me about a morning when the argument did not happen. What were you doing differently? Who noticed?" The answers are usually small, specific and unfamiliar to the family, which is precisely their value.

The miracle question. The model's signature technique, and the most misunderstood. Its purpose is not to elicit fantasy but to construct a detailed description of a life without the problem, in behavioural and relational terms: who would notice, what they would see, what would happen first, what would happen next, what the family would do differently at breakfast. The description is then used to identify what is already happening in miniature, and to set a first step.

Scaling. A numerical device that converts vague progress into a comparison. "On a scale of nought to ten, where nought is the worst it has been and ten is the miracle, where are you today?" The number matters less than the question that follows: what would one point higher look like, and what have you already done that got you to this number rather than lower? The second question is the one trainees forget, and it is the one that produces agency.

Coping questions. Used where the family's situation is objectively severe — bereavement, chronic illness, poverty, displacement — and where asking about miracles would be an insult. "With everything you have carried this year, how have you managed to keep the children in school?" The question presumes competence and produces evidence of it, and it is the model's instrument for the families that its critics say it ignores.

Small, observable change. The model's theory of change, in one sentence: a small difference, noticed and repeated, becomes a larger difference, because the family's description of itself changes. The therapist therefore works in units of one point on a scale and one behaviour at a time, and resists the family's invitation to plan a transformation.

"If it works, do more of it; if it does not, do something else." The model's pragmatism, stated as a rule. It frees the therapist from theory-driven fidelity and it imposes a discipline: the therapy must notice what works and stop what does not, which requires measurement rather than conviction.

Non-normativity. The model does not hold a theory of healthy family functioning. It does not say what a family should look like; it asks what this family wants and what is already moving in that direction. That position is the reason the model is used in child protection, schools and coerced settings, and it is also the reason its critics distrust it.

The brief format. Typically three to six sessions, with the first session doing most of the work. The model's brevity is not rationing; it is a claim that the useful conversation can be short if it is organised around solutions. The claim is supported by trials showing brief formats performing comparably to longer treatments for some presentations, and it is not supported for all (see 10.11).

10.2a Two transcripts of the same first ten minutes

The comparison that teaches the model faster than any description. Both are responses to a mother who says: "my son will not go to school and my husband says it is my fault."

Transcript A — problem-organised.

Therapist: When did this start? Mother: September, when he moved up. Therapist: And what do you think caused it? Mother: He was bullied, I think. Therapist: Did you tell the school? Mother: They did nothing. Therapist: And your husband blames you? Mother: He says I am too soft. Therapist: Are you? Mother: (pause) I do not know any more. Therapist: Tell me about your own childhood.

What was produced: a history, a hypothesis, a defensive mother, and a list of failures by the school, the husband and herself. The therapist is well-informed and the mother is worse off than when she arrived.

Transcript B — solution-organised.

Therapist: What would you like to be different by the end of our work together? Mother: He would go to school without a fight. Therapist: And what would that morning look like — the first ten minutes? Mother: I would wake him once, he would get up, and we would not shout. Therapist: Has there ever been a morning like that, even one? Mother: (pause) Last Tuesday, actually. Therapist: What was different about Tuesday? Mother: I had laid his uniform out the night before, and I did not go in three times. Therapist: So on Tuesday you prepared the night before and you went in once. What did that do? Mother: He was up before I shouted. Therapist: Whose idea was the uniform? Mother: Mine. Therapist: So you have already found something that works. What would it take to do that on three mornings this week?

What was produced: a goal in behavioural terms, an exception, the mother's own agency, and a task she designed. The therapist knows almost nothing about the history and the mother leaves with a plan.

The fair comparison. Transcript A is doing something B is not: assessing. It obtained the bullying information, which matters. Transcript B is doing something A is not: constructing. Its weakness is that the bullying is not mentioned, and if the mother had not volunteered it the therapist would not have known. The competent practitioner runs B's structure and inserts A's assessment where risk requires it — which is exactly what 10.7 specifies.

The exercise. Read both transcripts and mark, in transcript B, the four points where a less experienced therapist would have asked a problem question. Then write the sentence you would have used, and decide whether it was necessary.

10.2b The questions this model is actually made of

A repertoire list, because the model is learned as questions rather than as theory. Fifteen questions, grouped by function, each with the situation in which it earns its place.

Goal questions. "What would you like to be different by the time we finish?" — the opening. "And what would that look like on a Tuesday?" — the translation into behaviour. "Who would notice first, and what would they see?" — the relational frame.

Miracle questions. The eleven-question sequence (10.3a). The one to remember when time is short: "suppose this were solved while you slept — what would you notice first tomorrow?"

Exception questions. "Tell me about a time when it was better, even slightly." "What was different about that day?" "How did you make that happen?" "Who else noticed, and what did they say?" The third of these is the one that converts luck into agency.

Scaling questions. "Where are you today?" "What has kept it from being lower?" "What would one point higher look like this week?" "What would your husband say your number is?" — the fourth produces the disagreement that is often the session's most useful information.

Coping questions. "How have you kept going?" "How have you managed to get the children to school through all this?" "What have you held on to?" "Who has helped, and how?" These are the questions for severe, unchanging adversity, and they are the model's most humane instrument.

Difference questions. "What would your daughter say is different about you this week?" "What is different about the days when it is better?" "What is the difference between how you handled it on Tuesday and how you would have handled it a year ago?"

Ending questions. "What would tell you that you did not need to come back?" "How will we know the work is finished?" "What would be a good enough outcome, not a perfect one?"

The training rule. A trainee should be able to ask any of these fifteen questions without consulting a book, in the family's own language, and should know which three to abandon when the situation is severe. That is the whole of the model's technical content, and it is small enough to be mastered in a term — which is one reason the model is often taught badly, since it looks as though there is nothing to learn.

10.3 The session, in five moves

Move one — the goal (five minutes). "What would you like to be different by the end of our work?" The answer is converted into behavioural and relational terms: who would notice, what would they see, what would happen first. Vague goals ("I want him to respect me") are not rejected; they are translated ("what would he do that would show respect?").

Move two — the miracle question, or its equivalent (ten minutes). The full sequence: the miracle overnight; the first thing noticed tomorrow; who else notices; what they would see; what the family would do differently; what would be different at breakfast, at school, at bedtime. The sequence ends when the description is concrete enough for a stranger to film it.

Move three — scaling and exceptions (fifteen minutes). Where are you now, on nought to ten? What got you to this number rather than lower? What would one point higher look like? Has there been a time recently when the number was higher, and what was happening?

Move four — the compliment and the task (five minutes). The therapist offers a genuine compliment, grounded in what the family has said, and a task derived from the family's own material. The task is almost always observational — "notice the mornings that go better, and what you did" — or a repetition of an exception — "do on three mornings what you did on Tuesday".

Move five — the next appointment (two minutes). Often optional, left to the family, which is itself an intervention: the model's brevity is communicated by not assuming the family needs to return.

The structure's discipline. Each move has a time budget, and the therapist's failure mode is to spend the session in move one and two and never reach the task. A trainee should be able to complete the five moves in forty minutes, with ten minutes to spare for whatever the family needs.

10.3a The miracle question, built question by question

The technique fails when it is asked as a single question and the family's answer is left undeveloped. Eleven questions, in order, with what each one produces.

1. **The permission.** "I am going to ask you a strange question. Is that all right?" — produces consent and signals that the unusual is permitted.
2. **The miracle.** "Suppose tonight, while you are asleep, a miracle happens and the problem that brought you here is solved. You do not know it has happened because you are asleep. What is the first thing you notice tomorrow morning that tells you something is different?" — produces a concrete entry point.

3. **The first detail.** "What would you see in the room?" — produces sensory specificity.
4. **The first act.** "What would be the first thing you did differently?" — produces behaviour.
5. **The relationship effect.** "Who would be the first person to notice, and what would they see?" — produces the relational frame.
6. **The other partner's view.** "What would your husband notice about you that he has not seen for a while?" — produces the partner's perspective without asking the partner.
7. **The child's view.** "If your son were asked, what would he say was different about the house?" — distributes the description across the family.
8. **The hardest moment.** "What would be different at the part of the day that is hardest?" — prevents the description from being a pleasant morning only.
9. **The small piece already happening.** "Is any part of that happening already, even a little?" — the pivot to exceptions.
10. **The scale.** "On a scale of nought to ten, where nought is the worst and ten is the miracle, where are you today?" — produces a baseline.
11. **The first step.** "What would one point higher look like this week — something you could do, not something someone else would have to do?" — produces the task.

The discipline that separates the technique from a parlour trick. Every question after the second is asked of the family's answer, not of the therapist's imagination, and the therapist does not supply a detail the family has not produced. "Would you perhaps get up earlier?" is a failure: the family's miracle must be theirs.

The two questions that make it work in families. Questions 6 and 7. The miracle question is often taught as an individual technique; in a family session, asking each member what the others would notice turns it into a relational instrument and produces the session's most useful information about the family's shared hypothesis.

The counter-indications. Where a family is in acute crisis, where a member is dying, where the presenting problem is a recent bereavement, or where the family's situation is severe and unchangeable, the miracle question can be heard as an insult. Use the coping question instead, and leave the miracle for later or never.

10.4 Techniques, with the grammar of each

The miracle question. Eleven questions in sequence (10.3a). Grammar: permission, miracle, first sensory detail, first act, who notices, the partner's view, the child's view, the hardest moment, the piece already happening, the scale, the first step.

Scaling. Three questions, in order: where are you; what got you to this number rather than lower; what would one point higher look like. Grammar: the number, the credit, the step. The second question is the one that changes the session, because it obliges the family to describe what they have already done.

Exception questions. Four forms: "tell me about a time when the problem did not happen"; "what was different about that day?"; "who was there, and what did they do?";

"how did you make that happen?" The last form is the crucial one — it moves the exception from luck to agency.

Coping questions. "With everything you have faced, how have you managed to keep going?" and "how have you kept the children in school / kept the family together / got up in the mornings?" Used where the situation cannot be changed quickly, and used with respect rather than as a consolation prize.

The relationship question. "What would your wife say is the best thing you do as a father?" The question produces a competence description from a third position, and families find it easier to accept than a direct compliment.

The compliment, grounded. Not "you are doing a great job" but "you laid his uniform out the night before and you went in once instead of three times; that is a change you made and it worked." The compliment is evidence-based, attributed to the family, and specific enough to be repeated.

The observational task. "Between now and next time, notice the mornings that go better and what you did that made them better. Do not change anything; just notice." The task is deliberately modest, and its function is to train attention, which the next session can then use.

The repetition task. "Do on three mornings this week what you did on Tuesday." The task is derived from the family's own exception and is therefore both plausible and owned.

The "do something different" task. For a family stuck in a repetitive pattern: "next time the argument starts, do something different — anything, as long as it is not what you usually do — and notice what happens." The task's logic is that a change in one element of a loop changes the loop, and its risks are that it can look flippant if it is not framed carefully.

The letter or the note. A short written summary, given at the end of the session, containing the compliment, the family's own words about the miracle, and the task. The written form matters in families with low literacy only if it is read aloud and left with them; where literacy is limited, an audio message or a photograph of a whiteboard serves the same function.

The ending question. "What would tell you that you do not need to come back?" Asked at the first session, it communicates the model's brevity and gives the family a criterion for ending, which is itself an intervention.

10.4a A scaling exercise, worked in full

The technique's most portable instrument and the one most often performed perfunctorily. A full sequence with a family, so that the follow-up questions can be seen.

The family. The Okonkwos: mother Ada (34), father Chuka (39), Ebuka (9) defiant since the baby arrived. The presenting complaint is the evening.

The scale is defined first. "Nought is the worst evening you can remember — everyone shouting, Ebuka in the street. Ten is the evening you would like. Where is a normal

evening now?" Mother: "Three." Father: "Four, maybe." Therapist: "So you are not at nought, and you do not agree. That is useful."

The credit question. "What has kept it at three and not lower?" Mother: "I stopped shouting at bedtime, because it made him worse." Therapist: "That was your decision?" Mother: "Yes." Therapist: "And it worked?" Mother: "A bit." The exchange looks unremarkable and it has produced a self-attributed change, which is the technique's whole purpose.

The difference question. "When it is a four rather than a three, what is happening?" Father: "When I am home before seven." Therapist: "And what do you do when you are home before seven?" Father: "I take the baby so she can cook." Therapist: "So two things: you are home, and you take the baby."

The one-point question. "What would a four look like on a Tuesday when you are not home before seven?" Mother: "I would not be on my own with both of them." Therapist: "Who else could be there?" Mother: "My sister is two streets away." Therapist: "Has she ever been there on a Tuesday?" Mother: "Once, in June." Therapist: "What happened that evening?" Mother: (pause) "It was a four."

The task. "So the one point is not a new programme. It is your sister, on a Tuesday, once a fortnight, and your husband taking the baby when he is home. Which of those can you arrange this week?" The task is the family's, and the therapist has added nothing that the family did not say.

The review at the next session. "Where is the number now?" and, before the answer: "and what have you done since we last met?" The order matters: the credit question comes before the number, so that the family describes its own activity before it evaluates it.

The three errors to avoid. Error one: using the scale without defining the anchors, so that the number means nothing. Error two: asking "why not higher?", which is a criticism dressed as a question. Error three: accepting "a bit better" as the end of the work; the technique requires the specific behaviour, and the specific behaviour is where the task comes from.

10.5 The first session with the Adeyemis

The recurring family, in this model's hands, in forty-five minutes.

The opening (five minutes). "I have read the referral, and I would rather hear it from you. What would you like to be different?" Funke: "I want her back at school." Therapist: "And what would that look like on a Monday morning?" Funke: "She gets up when I call her, she eats something, and she goes." Therapist: "Who else would notice?" Funke: "My husband, on the phone. And Segun."

The miracle question (ten minutes). The overnight miracle, and the first thing Funke notices is that Amara is in the kitchen before her. Amara, asked directly, says she notices her mother not standing outside her door. Segun, asked what he would notice, says "nobody shouting before seven". The therapist writes all three answers on the sheet, in

the family's words. Tunde, on speaker, is asked what he would notice from Lagos, and says "the phone calls would stop being about trouble" — a sentence the therapist marks, because it is the first time the father has described what he wants rather than what is wrong.

The exceptions (ten minutes). "Has there been a morning lately that was closer to that?" Funke: "Thursday last week. She got up because Segun went in and told her there was a fight at school she wanted to see." Therapist: "So Segun went in, and she got up. Segun, what did you say?" Segun: "I said there was a fight and she would miss it." Therapist: "And it worked. Did you know it worked?" Segun: (smiles) "Yes." The exception is a sibling-mediated entry, which nobody in the family had identified as a resource.

The scaling (five minutes). "Nought is the worst week — three weeks ago, when she did not leave the room at all. Ten is the Monday you described. Where is this week?" Funke: "Two." Amara: "Four." Therapist: "Amara, what makes it a four?" Amara: "I went to the shop twice." Therapist: "Who knew that?" Amara: "Nobody." The therapist turns to Funke: "did you know she had been out twice?" Funke did not. The number disagreement is the session's most useful finding.

The compliment and the task (ten minutes). "What I have heard is this: you [Funke] have stopped standing outside her door in the morning, which is a change you made; Segun has found a way of getting her up that works, and he did not know it was a method; and Amara has been out of the house twice without telling anyone. My suggestion is that this week you notice — all three of you — the mornings that go better, and what happened the night before. And Amara, would you tell your mother when you go out, not to ask permission, just so she knows? That is the one thing I am asking."

The ending question. "What would tell you that you did not need to come back?" Funke: "When she has done a full week." Therapist: "So we will meet twice more and then decide."

The clinical note. The session contains no history of the school refusal, no exploration of the father's absence, and no formulation. It has a goal, three exceptions, a sibling resource, a task and an ending criterion. Whether that is enough depends entirely on what is not being asked — and the next section says what must be.

10.6 Case: what the model did not ask, and the session after

The model's critics are right that a solution-focused first session can leave a serious problem untouched. This section stages the consequence.

What was missed. At the second session, Amara says, in passing, that she is not going back to school "because of Mr Bello". The therapist asks one question — "what does Mr Bello do?" — and the family's account changes: a teacher who has twice humiliated her in front of the class, and a school that has done nothing. The school refusal is not a symptom of family organisation; it is a rational response to an unsafe classroom.

What the model would have done had it asked. A solution-focused therapist does not need a history to ask about safety; the questions are: "is there anything about school that is frightening, not just boring?"; "who at school knows about this?"; "what would have to

change at school for you to feel able to go?" The answers produce a second set of goals, addressed to the school rather than to the family, and the model's techniques — scaling, exceptions, small steps — work perfectly well on that target.

The general principle, stated as a rule. Before the first solution-focused question, the therapist asks the risk questions: violence at home, violence at school, self-harm, abuse, hunger, a child not in school and not safe. The model does not require a full psychosocial history, and it does require a screen. Trainees are taught the miracle question and not the screen, which is the training's failure rather than the model's.

The compromised first session, rewritten. The opening becomes: "before we talk about what you would like to be different, I want to ask two questions that I ask everybody. Is anyone at home being hurt? And is anyone at school or anywhere else being hurt or frightened?" Then the goal, the miracle, the exceptions. The screen costs ninety seconds and it changes the session's status from a conversation to a clinical contact.

What the second session did. The therapist went back to the school, with the mother's consent, and used the model at the boundary: the school's exceptions (which lessons Amara attended), the school's scale (how safe the classroom felt), and one small step (a change of seating and a named adult). Amara returned to school in week four. The family work continued for two more sessions. The model was not abandoned; it was extended.

10.7 Annotated transcript: the miracle question with a couple

Fourteen turns, from a first session with the Ogundipes, with the technique's grammar marked.

> **T1 Therapist:** I want to ask you both something unusual, and it will sound strange. Is that all right?
> **T2 Husband:** (laughs) Go ahead.
> **T3 Therapist:** Suppose tonight, while you are asleep, a miracle happens, and the thing that brought you here is solved. You do not know, because you are asleep. When you wake tomorrow, what is the first thing you notice that tells you something is different?
> **T4 Wife:** (long pause) He would be awake before me.
> **T5 Therapist:** And what would you see?
> **T6 Wife:** The kettle would be on. He never does that.
> **T7 Therapist:** So the kettle, and him awake first. What would you do then?
> **T8 Wife:** I would probably say something stupid like thank you.
> **T9 Therapist:** (to husband) And you — in this miracle, you are up first and the kettle is on. What are you doing?

> **T10 Husband:** I do not know. Reading the paper. Waiting for her to come out.
> **T11 Therapist:** Waiting for her. What does waiting look like?
> **T12 Husband:** (pause) I would be sitting at the table, not in the bedroom.
> **T13 Therapist:** So the difference is the table. (pause) On a scale of nought to ten, where nought is the worst it has been and ten is this morning with the kettle on, where are you today?
> **T14 Wife:** Two. (pause) But he made tea on Sunday.

Annotations. T1 is the permission, which is not a formality: the technique's strangeness is a real obstacle, and naming it makes it usable.

T3 is the miracle question itself, complete: the overnight frame, the explicit statement that they will not know, and the "first thing you notice" rather than "what would be different", which is the difference between a description and an abstraction.

T4 is a good answer and the therapist's discipline is visible at T5: she does not move to the next stage of the sequence. She asks for the sensory detail, and the detail — the kettle — is the material the whole session will use.

T7 asks for behaviour, which is the technique's third requirement. T8's self-deprecating answer is left uncontested; the therapist does not reassure her that it would not be stupid, because the answer is not the subject.

T9 turns to the husband, which is the move most trainees miss: the miracle must be described by both partners, and the husband's fantasy — the table rather than the bedroom — is a much more precise description of what he wants than anything he has said about the problem.

T11–T12 is a deepening question, used inside the miracle question, which is a legitimate combination: the technique produces a description, and the description can be deepened like any other material.

T13 is the pivot to scaling, asked too early by the textbook's standards and at the right moment here, because the description was concrete enough to anchor the ten.

T14 is the gift that the technique is designed to produce: "but he made tea on Sunday" is an exception, volunteered, and it makes the next fifteen minutes easy.

What the trainee should notice. The therapist asks eleven questions and offers no interpretation, no compliment and no advice until the end. The technique is almost entirely curiosity, organised in a particular direction — a direction the therapist chose, which is the model's hidden normativity and the subject of 10.10.

10.8 Laboratory: the miracle question and the scale

Sixty minutes, triads, two techniques and a critique.

Round one — the miracle question (twenty-five minutes). Eight minutes as therapist, following the eleven questions. The observer records: how many of the eleven were asked; whether the therapist supplied any detail the client had not produced; whether the sequence ended with a concrete first step.

Round two — the scale (fifteen minutes). Five minutes on the scale with the three questions — number, credit, one point higher — followed by writing the task in one sentence. The observer checks that the credit question came before the number at review.

Round three — the critique (twenty minutes). The triad takes the transcript from round one and answers three questions: what did the therapist fail to ask that a careful clinician would ask? what did the client say that was not followed up? and what would have happened if the client had disclosed harm?

Marking rubric.

Criterion	Weight	Full marks
Fidelity to the sequence	25%	All eleven questions present and in order; no supplied details
Concreteness	25%	Every answer converted into a filmable behaviour; sensory detail obtained
Credit and agency	20%	The family's own activity named before the number; task derived from their exception
Screen	20%	Risk questions asked before the first solution question, and recorded
Restraint	10%	No interpretation, no advice, no reassurance beyond the grounded compliment

The debrief question. "What did you want to say to the client and did not?" The answers are usually reassurances, and the discussion of why the therapist wanted to give them is the week's most useful hour.

10.8a A task designed for a family that has heard everything

The hardest audience for the model: a family referred at least twice before, who have been given advice, charts, contracts and a parenting programme, and who arrive expecting to be told what to do and to fail at it.

The first move with this family. Not the miracle question: the credit question. "You have been through three services and you are still here, on a Tuesday evening, for your son. What has kept you going?" The question is not flattery; it is accurate, and it produces the family's first experience of being described as competent rather than as non-compliant.

The second move. An audit of what has already been tried, conducted as a search for exceptions rather than as a critique of services. "Of everything you have tried, what worked for a while?" Families almost always name something — a chart that worked for a fortnight, a reward that worked for a month, a grandmother's intervention that worked twice. The therapist writes each item down and asks what the family did that made it work at all.

The third move, which is the task. "Of the things that worked a little, which one could you do again this week, for four days only?" The restriction to four days is deliberate: it is small, it is bounded, and it cannot be described as a regime. A family that has failed three programmes will do four days of something they chose.

The fourth move, the review. "You did four days. What did you notice?" The answer may be that nothing changed; the follow-up is "and what did you notice about yourselves doing it?", which is a different question and often produces a different answer.

What this sequence demonstrates. The model is not a set of techniques for naive families. It is a discipline for finding agency in families whose agency has been systematically described as deficient, by services as well as by themselves. Its instrument is the question that presumes competence, asked sincerely, once.

10.9 Six errors in solution-focused work

1. **The miracle question as a party trick.** Asked once, at speed, with no follow-up questions, and the family's answer left undeveloped. Fix: the eleven-question sequence, and a concrete first step at the end.
2. **No screen.** The session begins with goals and never asks about violence, self-harm or a child out of school. Fix: the ninety-second screen (10.6), asked of every family, every time.
3. **Solution talk imposed on grief.** A bereaved family asked what would be different if the problem were solved. Fix: the coping question, and permission to tell the story — the model's own literature allows both, and trainees forget the second.
4. **The compliment that flatters.** "You are wonderful parents" — unsupported, generic, and heard as management technique. Fix: the grounded compliment, built from something the family said.
5. **The task nobody designed.** "Notice what works" issued as the family leaves, with no specificity about what to notice. Fix: name the moments, the days, and who will report what.
6. **The therapy that cannot stop.** A model built for three to six sessions delivered for thirty, because the therapist enjoys the conversation. Fix: the ending criterion, asked in session one.

10.9a Three sessions where the model was the wrong choice

The bereaved family. A father, a mother and two children, three months after the death of a nine-year-old. The family's task is to grieve together, to tell the story of the death in a way that can be shared, and to reorganise a household that has lost a child. A solution-focused session would ask what they want to be different, and the only honest answer is "we want him back". The model is not wrong about families; it is wrong about this one, in this month. The correct response is a grief-informed family therapy, with the model's techniques available later, if at all.

The family in a child protection investigation. A family referred after a substantiated injury, in which the parents' account minimises the injury and a toddler is subject to a plan. A solution-focused session that begins with "what would you like to be different?" invites the parents to set the agenda, and the agenda they want is the removal of the social worker. The model's non-normativity, which is its virtue in voluntary work, is a liability where the state has a protective mandate. The correct response is a forensic and

protective process first, with the model's techniques used only after the safety questions are settled.

The family with a psychotic member. A young man in an acute episode, his parents frightened and exhausted. The miracle question presumes a shared reality within which a preferred future can be described; where reality is contested, the question can be confusing or distressing. The correct response is psychoeducation, medication liaison, and family work with a different evidence base (MFT 719), with solution-focused conversations reserved for the parents' own coping.

The general rule these three share. The model presumes a family that can name a preferred future, an agenda that is the family's to set, and a shared reality in which that future can be described. Where any of the three is absent, another model leads, and this one assists.

10.10 Culture, faith and the model's critique

Where the model travels well. Its respect for the family's own goals and its refusal to impose a theory of healthy family life make it unusually adaptable. In settings where families are wary of being judged — child protection, schools, immigrant families, families referred by a court — the model's non-normativity is a genuine asset. Its brevity suits families who cannot afford repeated appointments, and its conversational form suits families whose relationship to formal therapy is sceptical.

Where it strains. First, the language of individual goals can misdescribe a family whose decisions are made collectively. "What would you like to be different?" addressed to a young wife in a household where her mother-in-law governs the arrangements may produce an answer that is not hers to give. The model's individualism is implicit and real, and the correction is to ask the question of the family as a unit and to include the decision-makers.

Second, the miracle question is culturally specific in its optimism. Families living with chronic scarcity, political violence or displacement may find a question about overnight miracles naive, and the model's own literature acknowledges this: the coping question exists for exactly these families, and a trainee who knows only the miracle question has learned half the model.

Third, the model's refusal of a normative theory is not the same as neutrality about harm. A family's preferred future may include arrangements that are unjust — a daughter withdrawn from school, a wife's employment forbidden — and the model's non-normativity can leave the therapist without grounds to raise it. The grounds are professional, not theoretical: the legal and ethical obligations of the practitioner override the model's preference for the family's agenda.

Faith. The model is compatible with religious families and is often used well with them, because the family's own values carry the goals. A family whose preferred future is "we pray together again" gives the therapist a goal that is entirely theirs. The two errors are symmetrical: treating the family's faith as a resource to be exploited for compliance, and treating it as irrelevant to the goals the family names.

> **DEBATE**
> "Solution-focused therapy is either the most respectful model in this book or the one that most easily abandons the vulnerable. It cannot be both." Resolve the claim.

10.10a The two strongest critiques, and the model's answers

Critique one: it ignores power and risk. The model asks the family what they want and treats them as the expert. Where the family contains a coercive member, the person whose goals are being set is the person with the power, and the model's respect for the family's agenda becomes respect for the abuser's agenda. The same applies to a family whose "solution" is the withdrawal of a child from school, a marriage arranged against a daughter's wishes, or the concealment of an injury.

The model's answer. The practitioner's legal and professional duties are not suspended by the model; solution-focused therapists working in child protection use the model within a statutory frame, and the literature on "mandated clients" addresses exactly this. The model's techniques can be used to engage a family in a process whose goals are not entirely theirs.

The fair verdict. The answer is adequate in principle and depends entirely on the practitioner's competence. The critique identifies a real hazard for the model's enthusiasts and a real strength for its cautious users.

Critique two: the evidence is narrower than its reputation. Solution-focused brief therapy is widely described as evidence-based, and the evidence is real but uneven: reasonable support for depression and for some child and adolescent presentations, weaker and sometimes null findings for others, small samples in many trials, and a literature that includes enthusiastic small studies with weak designs. The reputation has outrun the trials in some settings.

The model's answer. The approach is brief and cheap, which makes it attractive to services, and its evidence is comparable to other brief approaches for the same presentations. Where it performs less well — complex trauma, severe mental illness, some family presentations — other models lead.

The fair verdict. The critique is correct and is the reason this week's evidence note is cautious. The model should be described as "supported for specific presentations in brief formats" rather than as a general-purpose first-line family therapy.

10.11 Evidence note: real, uneven, and often oversold

What exists. Randomised trials of solution-focused brief therapy exist for depression, for some anxiety presentations, for child and adolescent behaviour problems, and for outcomes in school and social care settings. Several meta-analyses report small to moderate effects, with wide confidence intervals and substantial heterogeneity. Trials in family formats are fewer than trials in individual formats, and the family evidence is thinner than the model's reputation suggests.

What the evidence shows about mechanism. The model's proposed mechanisms — the construction of a preferred future, the identification of exceptions, the amplification of agency — have some process support, and the common factors literature (Week 11) offers an alternative explanation: the model may work largely through alliance, expectancy and the therapist's optimistic attention. The model's own theorists would not object to that explanation.

What the evidence does not show. That it outperforms other brief approaches for the same presentations; that it is safe or effective where coercion, violence or acute risk is present; that brief formats are as good as longer ones for complex family difficulties; and that the model's techniques are separable from the therapist's manner, which is the practical question for training.

The matrix cell. "Small to moderate randomised support for depression and some child and adolescent presentations in brief formats; family-format evidence thinner; mechanisms plausibly shared with common factors; not indicated where coercion, acute risk or unresolved grief dominates."

The practitioner's claim. "I use solution-focused work where the family has a goal they can name and no acute risk, because it is brief, respectful and supported for some presentations; I screen first; I use the coping question rather than the miracle question where the situation cannot change; and I measure outcome because the evidence is uneven." That sentence is defensible and it is honest about the model's limits.

10.12 Key terms

Term	Definition
Solution talk	Conversation about preferred futures, exceptions and small steps
Problem talk	Conversation about causes, history and blame
Miracle question	A constructed description of life without the problem, in behavioural detail
Scaling	A nought-to-ten comparison producing credit, baseline and a next step
Exception	A time when the problem was absent or smaller
Coping question	An enquiry into how a family has managed despite severe adversity
Client as expert	The family holds the knowledge; the therapist holds the questions
Grounded compliment	A compliment built from specific evidence the family supplied
Observational task	"Notice what works" — training attention before changing behaviour
Repetition task	Doing more of what already worked
Ending criterion	The family's own answer to "how will we know we are finished?"
Non-normativity	The model's refusal to hold a theory of healthy family functioning

10.13 Self-test

1. Distinguish solution talk from problem talk, and state what the distinction does not claim.
2. Give the eleven questions of the miracle sequence, in order.
3. What are the three scaling questions, and which one produces agency?
4. When is the coping question used instead of the miracle question, and why?
5. What is the ninety-second screen, and why must it come first?
6. Name the three families for whom this model is the wrong choice.
7. State the two strongest critiques and the fairest verdict on each.
8. What does the evidence support, and what does it not?

Answers. 1. Solution talk concerns preferred futures, exceptions and next steps; problem talk concerns causes, history and responsibility. The distinction does not claim that problems are unreal or that the family's pain should be ignored; it claims that describing a problem does not describe its solution. 2. Permission; the miracle overnight; the first thing noticed; what you would see; the first thing you would do differently; who notices first and what they see; the partner's view; the child's view; the hardest moment; the piece already happening; the scale. (The first step is the twelfth question in practice.) 3. Where are you; what got you to this number rather than lower; what would one point higher look like. The credit question produces agency. 4. Where the situation is severe and unchangeable — bereavement, chronic illness, poverty, displacement — because a miracle question would be heard as an insult and the coping question produces evidence of competence and survival. 5. A short set of risk questions — violence at home, violence elsewhere, self-harm, abuse, a child not in school and not safe — asked before the first solution question, because the model's structure will not otherwise produce them and the practitioner's duty is prior to the model. 6. A recently bereaved family; a family in a child protection investigation where the account minimises harm; a family with a member in an acute psychotic episode. 7. Critique one — it ignores power and risk and can adopt the coercive member's agenda; fairest verdict, adequate in principle and dependent on the practitioner's competence. Critique two — the evidence is narrower than the reputation; fairest verdict, correct, and the reason to describe the model narrowly. 8. Small to moderate randomised support for depression and some child and adolescent presentations in brief formats; not established as superior to other brief approaches, not established for acute risk or complex trauma, and not separable from common factors.

10.14 Reflective task

Take a difficulty in your own life that has lasted more than a year. Answer the eleven miracle questions about it in writing, taking a full twenty minutes. Then answer the credit question: what has kept this from being worse? Most students find the exercise uncomfortable and then useful, and the discomfort is the point: it shows you what the model asks a family to do in the first ten minutes of a session, with a stranger, about something worse.

10.15 Readings for this week

- **de Shazer (1985)**, Keys to Solution in Brief Therapy, the founding text, read for the questions rather than the theory.
- **Berg & Miller (1992)**, Working with the Problem Drinker, the model applied to a hard presentation.
- **Berg (1994)**, Family-Based Services, the model in child protection and mandated settings, the most useful text in this list for public-sector practice.
- **De Jong & Berg (2013)**, Interviewing for Solutions, the training manual, with transcripts.
- **Gingerich & Peterson (2013)**, the meta-analysis, for the outcome claims.
- **Franklin et al. (2012)**, the review of school-based applications, for the evidence outside clinics.

Read Interviewing for Solutions with a highlighter, marking every question the therapist asks; the model is best learned as a repertoire of questions rather than as a theory. For the essay, describe the evidence as "small to moderate, heterogeneous and mostly individual-format" and then say what that means for your practice. The reading-log question this week is: "what does this model refuse to know, and what happens to a family because of that refusal?"

Next: Week 11 — Narrative therapy

Week 11 — Narrative therapy

The question. Which story is dominating this family's life, and who wrote it?

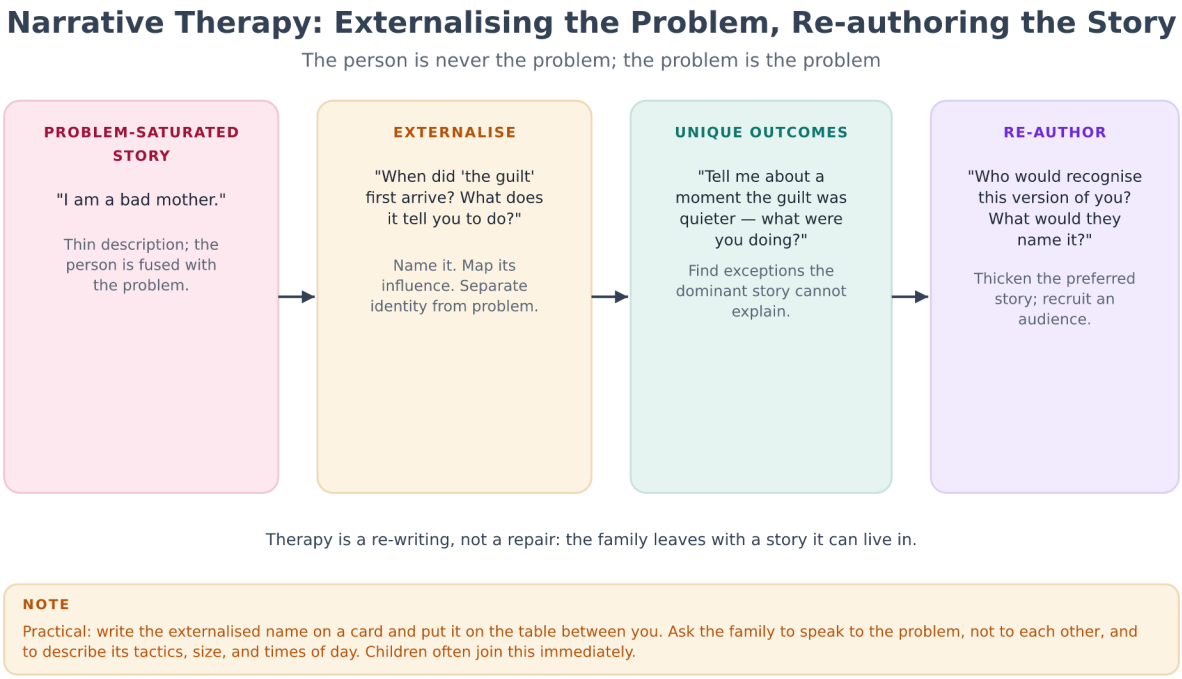


Figure 11.1 — Externalising and re-authoring the preferred story

Core concepts. The person is not the problem; the problem is the problem; dominant and preferred stories; thin versus thick descriptions; unique outcomes; externalising conversations; re-membering; definitional ceremonies; the therapist as decentred but influential.

Signature techniques.

- **Externalising:** "When did 'the guilt' first arrive in your life? What does it whisper to you?"
- **Mapping the influence:** "When does it have the most power? When does it lose power?"
- **Unique outcomes:** "Tell me about a moment when 'the shouting' was quieter."
- **Re-authoring:** "If you were to name this new version of yourself, what would you call it? Who would recognise it?"
- **Letter-writing** and **outsider-witness groups** to thicken the preferred story.

Contribution. The most culturally adaptable of the schools; central in work with children, with communities, and with people who have been defined by a diagnosis.

Blind spots. Can under-attend to material reality and risk; the linguistic sophistication can be inaccessible; evidence base is growing but smaller than EFT or CBT.

CASE

A mother referred by the school as "negligent". The therapist does not argue with the label but asks when "the exhaustion" took over, what it makes possible, what it prevents. Within thirty minutes the mother has described three occasions when she "fought the exhaustion and won", including a week when she arranged a rota with her sister. The referral, still true, is no longer the whole story.

11.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Why stories	The narrative premise, and the politics behind it	Name a label that has been applied to a family they know
10–25	Externalising	The exhaustion case, demonstrated with a volunteer	Rewrite three internal descriptions as external
25–40	Mapping the influence	Two columns: what it makes possible, what it prevents	Map one problem's influence
40–55	Unique outcomes	Finding a moment outside the dominant story	Identify three unique outcomes in a transcript
55–70	Re-authoring and naming	The preferred story, given a name by the family	Name and thicken one preferred story
70–80	Documents and witnesses	Letters, certificates, outsider witnesses	Draft a four-line letter
80–90	Culture and evidence	The model's politics, its uses, its evidence	Fill the matrix row

The teaching decision that makes the week work. Narrative therapy arrives in the classroom trailing a reputation for linguistic cleverness, and the fastest way to destroy that reputation is to show a session in plain language with a family who are not academics. The lecture should therefore use the week's own case — a mother labelled negligent by a school — and demonstrate the externalising conversation in ordinary sentences. The class should notice that the therapist never uses the word "narrative", never says "externalise", and never mentions Foucault.

The sentence the lecture must land. "The person is not the problem; the problem is the problem." A student who can say that sentence and then, in a role play, ask a mother when "the exhaustion" first arrived and what it whispers to her, has the model. A student who can only explain the theory has nothing.

11.2 Core concepts

Stories organise experience. The narrative premise: human beings make sense of their lives by telling stories, and a story selects — it includes some events, omits others, and gives the whole a plot with a character, a cause and an ending. A family's story about itself is not a neutral description of its history; it is a selection, and the selection has consequences, because people act according to the story they are living in.

Dominant stories. The account that has become authoritative — often because it was told by someone with power: a school, a clinic, a court, a grandmother, a diagnosis. The dominant story in the week's case is "negligent mother", and its authority comes from the school's referral. Dominant stories are not necessarily false; they are partial, and their partiality is what makes them oppressive.

Thin and thick descriptions. A thin description is a label: "negligent", "depressed", "defiant", "enmeshed". A thick description is a story with events, intentions, contexts and meanings. The model's work is the thickening of descriptions, and its technique is the question that asks for the particular: "when was the last time? what did you do? who saw? what did that mean to you?"

The person is not the problem. The model's foundational principle, and the one it shares with solution- focused work (Week 10) while deriving it differently. The problem is a separate entity with its own history, tactics and effects; the person is the person. The principle is not a kindness; it is a strategic move, because a person who is identified with a problem must defend themselves, while a person who is in a relationship with a problem can negotiate.

Externalising. The conversational practice that separates the problem from the person and gives it a name, a shape, a history and an influence. "When did the exhaustion first arrive? What does it make you do? What does it whisper? When does it lose its grip?" The grammar of externalising is consistent: the problem is a subject that acts, and the person is an agent who has a relationship with it.

Mapping the influence. Two enquiries, in order. First, the influence of the problem on the person's life: what it makes possible, what it prevents, who it has recruited, what it has cost. Second, the influence of the person on the problem's life: when it has been resisted, reduced, escaped, outwitted. The first enquiry establishes the problem's reality and seriousness — which is why the model is not naive optimism — and the second establishes the person's agency.

Unique outcomes. Moments that fall outside the dominant story: the morning the shouting was quieter, the week the mother arranged a rota, the day the boy went to school. The term replaced "exception" in the narrative literature to mark a difference of purpose (exceptions feed a solution; unique outcomes feed a story), and the technique is careful about not claiming too much: a unique outcome is not proof of progress, it is a crack in a story, and the therapy's work is to widen it.

Re-authoring. The construction of a preferred story from the unique outcomes: what the person did, what it says about what they value, who would recognise it, what it would be called. The preferred story must be the family's, in the family's language, and it must be thick enough to be lived in — which is why the model asks for names, witnesses and documents.

Re-membering. Michael White's term for the deliberate inclusion of significant figures — living or dead, present or absent — in the construction of identity. "Who would have known you were capable of this? What would your grandmother say if she could see you?"

The technique is especially powerful in families where an important figure has died and the family's story has narrowed.

Definitional ceremonies and outsider witnesses. Structured occasions in which an audience — a team, a group, a carefully chosen set of relatives — witnesses the preferred story and reflects on what struck them, producing a public, shared definition of the person that competes with the dominant one.

Documents. Letters, certificates, declarations, written summaries given to the family. The document is not a clinical record; it is an artefact of the preferred story, and its function is to survive the session and be re-read at home.

The therapist as decentred but influential. The model's position on power: the therapist's influence is real and cannot be neutralised, so it must be exercised in the service of the family's own authorship. The therapist asks the questions and does not supply the story; the therapist's interest shapes what is noticed, and the model's ethic is to make that shaping visible and to hand the authorship back.

The politics. Narrative therapy emerged partly as a critique of the power exercised by mental health systems over the people they describe. Its refusal to diagnose, its interest in the effects of labels, and its work with communities and with people who have been defined by a diagnosis all follow from that critique. That origin is the source of its cultural adaptability and of its evidentiary difficulty (see 11.10).

11.2a An externalising conversation, line by line

The technique taught as a sequence of eight moves, with the sentences that perform each one, and a family that is not academic.

The family. Mrs Bello, 38, referred by the school after her son missed eleven days. The referral says "negligent". She has three children, works shifts at a market stall, and her husband died two years ago.

Move one — find the name in the family's language. "The school has written something about you that I want to leave outside this room for now. What I want to understand is what it has been like at home. If the thing that has been making the mornings impossible had a name, what would you call it?" She says: "tiredness. But it is more than tiredness. It is like a heaviness." The name is hers, not the therapist's.

Move two — personify, gently. "This heaviness. When did it first come into your life?" She says: "after my husband died, but it got worse when I started the early shift." The question gives the problem a history, which is the first step toward its being an entity rather than an identity.

Move three — map its influence on her. "What does the heaviness make you do?" She says: "it makes me sleep through the alarm. It makes me lie to the school." "And what does it make you feel about yourself?" "That I am a bad mother." The therapist writes the phrase down. This enquiry is not cruel; it is the technique's respect, because it takes the problem's power seriously.

Move four — map its influence on the family. "Who else does the heaviness affect?" She says: "my daughter, she takes the little ones to school now." "And what does it make her do that she would not otherwise do?" "She has grown up too fast." The mapping is now systemic, and the family's own observation has become part of the description.

Move five — turn the enquiry. "I want to ask you the opposite question now. When has the heaviness been less strong? When have you had the upper hand over it?" She says, after a pause: "the week my sister did the school run." The unique outcome has arrived, and the therapist does not pounce on it.

Move six — thicken it. "Tell me about that week. What did you do to arrange it?" She says: "I asked her. I never ask anyone." "What did asking require of you?" "Pride. I had to swallow it." The unique outcome now has a name (asking), a cost (pride) and an agent (her).

Move seven — name what it says about what she values. "You swallowed your pride so that your children would get to school. What does that tell you about what matters to you?" She says: "that they come first. That I am not what they wrote." The sentence is the preferred story's first line, and it is entirely hers.

Move eight — plan the thickening. "What would it take to ask again, this time of a neighbour?" and then the document: the therapist writes a short letter, in her words, describing the week she asked her sister and what it cost her, and gives it to her to keep. She reads it twice before leaving.

What the sequence does and does not do. It does not deny the eleven missed days, the school's concern, or the school's duty; the letter is copied to the school with Mrs Bello's consent, and the school's safeguarding process continues. It does produce a description in which she is an agent with a history of resistance, which is the model's contribution to a case that other models would describe only as a deficit.

11.3 The therapy: what a narrative session looks like

The structure is loose and the sequence is not. A narrative session does not follow a five-move protocol; it follows a conversational logic: the problem is externalised, its influence is mapped, the person's influence is mapped, unique outcomes are found and thickened, and the preferred story is named and witnessed. The moves can occur in any order, and a session may spend forty minutes on one of them.

Session one. The problem is named and externalised; its influence is mapped on the person and on the family; the family's influence on it is mapped; one or two unique outcomes are found; and the session ends with a question to take away rather than a task: "notice this week when you have the upper hand, and what you do."

Session two. The unique outcomes reported are thickened; the preferred story begins to be named; and the family is asked who else would recognise this version of them. The therapist may propose a witness — a relative, a friend, a member of a team — with the family's consent.

Sessions three to five. The preferred story is built and documented; the family's relationships are re-membered (who knew, who would know, who is missing from the story); the practical problems that were present at referral are addressed, now inside a story in which the family is not the problem; and the ending is planned.

The typical course. Five to ten sessions. Narrative therapy is brief in practice, though its literature is not written in terms of session counts, which is itself an expression of the model's non-manualised character.

What the therapist brings to each session. Curious questions, a notebook, and the discipline of not knowing the family's story in advance. The therapist's own theoretical language is kept out of the room; the family's words are used and are written down, because the words are the material from which the preferred story will be built.

The risk the therapist manages. That the conversation becomes an exercise in clever reframing, in which the family's pain is translated into a story that flatters the therapist's skill. The safeguard is the mapping of influence: the problem's reality and cost are established in full before any unique outcome is sought, and a family who have not been allowed to say how bad it is will not have a story worth building.

11.3a Mapping the influence, in two columns

The technique that carries the model's seriousness, and the one most often skimmed by trainees anxious to reach the unique outcome.

The exercise. On a sheet, two columns. Left: the problem's influence on us. Right: our influence on the problem. The family fills the left column first, in as much detail as they can manage, and the therapist prompts with five questions: what does it make you do; what does it stop you doing; what does it make you think about yourself; what does it do to your relationships; what has it cost?

A worked example. The Adeyemi family, working with "the worry" (their name for the anxiety that keeps Amara at home).

Left column. It makes Amara check the door twice. It stops her going to the market. It makes her mother telephone the school every morning. It has cost Funke her job's leave allowance. It makes Segun sleep in his mother's room. It whispers to Amara that something will happen to her mother if she leaves.

Right column. The night Amara went to the shop. The morning Segun made her laugh and she ate breakfast. The week Funke stopped answering the school's calls and wrote a letter instead. The evening the family prayed together and Amara slept without checking the door.

What the columns produce. A description in which the problem is real, has tactics and costs, and is not invincible. The right column is thin at first, which is normal and instructive: the family's attention has been organised by the left column for months, and the therapist's questions are building the missing account.

The discipline. The left column is not hurried. A therapist who moves to the right column after two answers will produce a family who feel managed, and the unique outcomes that

follow will be the therapist's rather than the family's. Fifteen minutes on the left column is not wasted time; it is the condition of the work.

The written artefact. The two columns, copied in the family's words, go home on one sheet. It is the therapist's most useful document in this model, and it is more persuasive than any explanation of the technique.

11.3b The notebook, and why it is a technique

Narrative therapists write in the session, and the writing is not administration. Four uses, each with a sentence the therapist can say.

Use one — the family's words, captured. The therapist writes the family's phrases as they are spoken and then reads them back: "you said the heaviness whispers that you are a bad mother. Did I get that right?" The reading back is a technique in its own right: it demonstrates that the words were heard exactly, and it converts a spoken phrase into a record. Families frequently correct the therapist's version, and the correction is more valuable than the original.

Use two — the two columns, on the page. The mapping is written where the family can see it, and at the end of the enquiry the page is shown to them. A family who have never seen their situation described as two columns often respond to the shape of the page before they respond to the content: the left column is long and the right column is short, and the disproportion is the session's argument.

Use three — the document, drafted in front of them. A letter written in the session, in the family's presence, is a different object from a letter typed afterwards. The family watch their own words becoming a document, they correct it as it is written, and they leave with something that was made in the room. The therapist should read it aloud before handing it over, and should ask whether anything should be changed.

Use four — the following session's opening. The notebook's first page for each family holds the phrases that were promised a follow-up. The session opens with them: "when we last met you said you would notice the mornings when you had the upper hand. What did you notice?" The notebook makes continuity visible, which matters in a model whose sessions are conversational and could otherwise drift.

The caution. The notebook must not become a file. A family who see the therapist writing a description of them have the same experience as a family who read a referral, and the therapist's discipline is to write only what the family would recognise, to show the page, and to say what it is for. That discipline is the model's ethic applied to its own paperwork.

11.4 Techniques, with the grammar of each

Externalising. Eight moves: name in the family's language; give it a history; map its influence on the person; map its influence on the family; turn the enquiry; thicken a unique outcome; name what it says about what the person values; plan the thickening and record it.

The problem's name. Insist gently on the family's word. "The heaviness", "the worry", "the shouting", "the guilt", "the black dog". The name must be expressible in a sentence

with a verb — "the shouting makes me leave the room" — which is the test of whether it has become an entity.

The influence questions. Five, in order: what does it make you do; what does it prevent; what does it make you think about yourself; what does it do to your relationships; what has it cost.

Unique outcome questions. "Tell me about a time when it was not in charge." "What were you doing?" "How did you manage that?" "Who would have noticed?" "What does that say about what you want?" The last question is the bridge to re-authoring.

Re-authoring questions. "If this version of you had a name, what would it be?" "Who would recognise it?" "What would they say they see in you?" "What would be the next chapter?"

Re-membering questions. "Who would have known you could do that?" "What would your mother say if she were here?" "Who is missing from this story who should be in it?" Used with care where the missing person's absence is itself the wound.

The letter. Four lines: what the therapist witnessed; the family's own words about the moment; what it says about what they value; and a question for the next meeting. Signed, dated, given, and copied where appropriate to a professional who needs to know (with consent).

The certificate. A formal document, invented with the family, marking a development the family names — a year without a particular argument, a child's first full week at school. The certificate works because it is slightly absurd and entirely serious, and because it can be shown to relatives.

The outsider witness. Three or four people, chosen with the family, who listen to the preferred story and then respond in a fixed sequence: what struck me; what image stayed with me; what does this say about the person; and why does it matter to me. The sequence prevents advice and produces a public thickening.

The absent but implicit. A technique for finding what matters in what has been lost: "you have spoken about the absence of your father. What does the ache tell you about what a father should be?" The question reads a value from a gap, and it is one of the model's most delicate instruments.

11.5 The first session with a family who have been described

The model's signature presentation: a family who arrive with a professional description already attached.

The referral. The Okonkwo family: Ebuka (9), described by two schools as "defiant and possibly traumatised"; Ada (34), described in the referral as "overwhelmed, possibly depressed"; Chuka (39), described as "disengaged". The family have read none of this, and the social worker has summarised it to them in one sentence: "they think we are failing him."

The opening. "I have a referral here with several descriptions in it. Before we talk about anything in it, I want to know how you would describe what has been happening — in your own words, and I will write them down." The sentence does three things: it names the presence of professional descriptions, it declines to endorse them, and it hands the authorship over immediately.

The naming. The family's word for the problem is "the fighting". Not defiance, not trauma: the fighting. "The fighting. When did the fighting come into this house?" The therapist uses the family's word from then on and never uses the referral's.

The mapping. What does the fighting make you do? Ada: "it makes me shout and then it makes me cry in the bathroom." What does it do to your relationship with Ebuka? "It makes me not want to be near him." What does it do to you and your husband? "It makes us not speak, because we blame each other." What has it cost? "It has cost me my confidence as a mother, and it has cost him his school."

The turn. "When has the fighting been weaker? When have you had the upper hand?" Ada: "last weekend, when his cousin came. He was different for two days." "What were you doing?" "I was not on my own."

The thickening. "So when you are not on your own, the fighting is quieter. Who could you not be on your own with?" Ada names her sister. "What would it take to ask her?" and then the objection: "she has her own children." The therapist does not solve the objection; she writes it down and asks what Ada has done in the past when she has had no help, which produces a second unique outcome: the fortnight when she organised a rota.

The naming of the preferred story. "There seem to be two descriptions of you in this room. One is in the referral. The other is a woman who, when she had nothing, organised a rota. Which one do you want to build on?" Ada says: "the second one. But I do not believe it most days." The honesty is important, and the therapist does not argue with it: "then let us make it thick enough to believe."

The document. A four-line letter, written in the session, in Ada's words, describing the rota and what it required of her. She takes it home. At the second session she says she has read it three times and shown it to her sister.

What the session did not do. Did not assess Ebuka's trauma; did not ask about the paternal grandmother's death; did not formulate; did not give advice. It established the family as the authors of their own description, which is the model's precondition for everything else.

11.6 Case: the mother described as negligent

The week's case, worked in full, with the referral's language tracked through the therapy.

The referral. "Mrs Bello, 38, is failing to ensure her son's attendance. She presents as disorganised and possibly depressed. A parenting assessment is recommended."

The first session. The therapist opens by asking what the family would call the problem if they could name it, and the name that emerges is "the heaviness". The referral is not discussed. The mapping takes thirty minutes, and it is detailed and grim: the heaviness

makes her sleep through alarms, lie to the school, and believe she is a bad mother; it has recruited her daughter into the school run; it has cost her the trust of her son, who now hides his school shoes.

The turn. "When has the heaviness been less strong?" Three answers emerge: the week her sister did the school run; the morning her son made her laugh and she got up; the day she went to the market early and sold everything before nine.

The thickening. Each answer is developed: what she did, what it required, who noticed, what it says about what she values. The third answer produces the richest material: she is good at her stall, she is known for it, and the early mornings are the one place where the heaviness has never had the upper hand.

The preferred story. The therapist asks: "if there were a description of you that was truer than 'negligent', what would it be?" Mrs Bello says: "a woman who gets up at four". The therapist writes it down and the phrase is used for the rest of the therapy.

The practical work, inside the story. The attendance problem is not ignored. A plan is built from the preferred story: the early mornings are her strength, so the school run is reorganised around them — the children go with her to the stall before school, and a neighbour brings them the last stretch. The plan is hers, it uses her competence rather than her deficit, and it is the kind of plan that a parenting programme would not have thought of.

The school. With consent, the therapist writes to the school, not to dispute the referral but to add a description: "Mrs Bello gets up at four and has run a stall for eleven years; the attendance plan built on that has produced nine consecutive days of attendance." The school's file now contains both descriptions.

The outcome. Attendance recovers in six weeks. The heaviness does not disappear, and the therapy does not claim that it has; it is named, mapped, and reduced in influence in the mornings, and a referral for bereavement work is made for the underlying loss, which the model has been careful not to claim as its own territory.

What this case teaches. That externalising is not a denial of the referral's content — the child had missed eleven days, and the school was right to be concerned — but a refusal to let the referral be the whole description. That the preferred story can be practical rather than poetic: "a woman who gets up at four" is a story with a plan inside it. And that the model's ethics are enacted in the correspondence: the therapist did not argue with the school, and did not let the school's account go unchallenged.

11.7 Annotated transcript: re-authoring with a young person

Sixteen turns with Amara Adeyemi (14), seen alone for one session within the family work, because the family's story about her had become fixed.

> **T1 Therapist:** Your mother has a name for the thing that keeps you at home. She calls it "the worry". Is that your name for it?
> **T2 Amara:** (shrugs) I do not call it anything.
> **T3 Therapist:** If you had to, what would you call it?
> **T4 Amara:** (long pause) The what-if.
> **T5 Therapist:** The what-if. What does the what-if ask you?
> **T6 Amara:** It asks what if something happens to her while I am at school.
> **T7 Therapist:** And what does it make you do?
> **T8 Amara:** It makes me stay. It makes me check the door. It makes me text her.
> **T9 Therapist:** What does it stop you doing?

> **T10 Amara:** Everything. Netball. My friends. It stopped me going to Tola's birthday.
> **T11 Therapist:** I want to ask you a different question. In the last month, was there a day when the what-if was quieter?
> **T12 Amara:** (pause) The day of the fight at school. When Segun came in.
> **T13 Therapist:** What was different about that morning?
> **T14 Amara:** I wanted to see it. I forgot to ask the what-if.
> **T15 Therapist:** So on that morning, something you wanted was louder than the what-if. What was it?
> **T16 Amara:** (small pause) That I wanted to be there. With everybody.

Annotations. T1–T4 is the naming, and the therapist's question is careful: she offers the mother's name and asks whether it is Amara's. Amara's own name — "the what-if" — is better than anything the therapist could have supplied, and it is the name that will be used.

T5 is the personification: the problem becomes a subject that asks questions, which is a more usable formulation for a fourteen-year-old than "anxiety".

T7–T10 is the mapping, in the two forms that matter for a young person: what it makes you do, and what it stops you doing. The second is the one that produces the cost, and T10's list — netball, friends, a birthday — is the material the therapy needs.

T11 is the turn, and the trainee should notice the framing: not "have you ever felt better?" but "was there a day when it was quieter?", which presumes that such days exist without asserting that they were victories.

T12–T14 produce a unique outcome that is specific and testable: the morning of the fight, when curiosity was louder than fear. T14's phrase — "I forgot to ask the what-if" — is the model's whole theory of change in seven words, and the therapist's response at T15 is to treat it as information rather than as a breakthrough: "something you wanted was louder". The formulation keeps Amara as the agent and the what-if as the other party.

T16 is the preferred story's first sentence: "that I wanted to be there, with everybody". It is modest, credible, and it belongs to her.

What the therapist did not do. Did not reassure her that her mother would be safe (which would have been an argument with the what-if); did not praise her (which would

have made the therapist the author); did not interpret the what-if as separation anxiety; and did not ask about her father, which the family work would address separately.

11.7a Three externalising conversations that failed

Failure one — the problem without a cost. A trainee externalised "the anger" with a father in the first five minutes, moved straight to unique outcomes, and produced a session in which the father described three occasions of self-control and left without ever having said what the anger had done to his family. The wife did not speak. The cause: the left column was skimmed. The model's seriousness lives in the mapping, and a technique that skips it becomes a technique for managing the therapist's discomfort.

Failure two — the polite problem. A trainee named the problem "the communication difficulty", a phrase no family uses. The family agreed politely and the conversation went nowhere, because the name had no life in it. The cause: the therapist supplied the name. The test is the family's own sentence: "the communication difficulty makes me leave the room" is not a sentence anyone says.

Failure three — the story that indicted someone. A mother and a fifteen-year-old externalised "the lying" and the conversation became, within ten minutes, an account of the adolescent's deceit, with the mother as witness and the therapist as examiner. The cause: the problem was named from one member's perspective, and the therapist did not check that the name was shared. The correction: the naming question is asked of every member — "if this had a name, what would each of you call it?" — and the difference between the names is itself the material.

The general lesson. Externalising requires a name the family recognises, a mapping that takes the problem seriously, and a check that nobody in the room has been cast as the problem by the naming itself.

11.8 Laboratory: externalising, mapping and a letter

Sixty minutes, triads, three tasks and a written artefact.

Task one — the naming (ten minutes). One student describes a current difficulty in their own life; the therapist elicits a name in the client's language and tests it in a sentence with a verb. No further work is done. The debrief asks what made the name usable.

Task two — the two columns (twenty-five minutes). The mapping, with fifteen minutes on the left column before any turn is attempted. The observer times the columns and counts the prompts used.

Task three — the letter (twenty-five minutes). A four-line letter, written in the client's words, and read aloud. The observer checks: is it in the client's language? does it include a witnessed moment? does it say what the moment reveals about what the client values? does it end with a question?

Marking rubric.

Criterion	Weight	Full marks
Naming	20%	Name in the family's language, usable in a sentence, agreed by all present
Mapping the influence	30%	Left column completed in depth before any turn; at least five prompts used
Unique outcome and thickening	30%	Specific, her own action, what it required, what it says about her values
Document	20%	Four-line letter in the client's words, witnessed, dated, usable at home

The debrief question. "What did it cost you to stay in the left column?" Trainees report discomfort, boredom and a wish to help; the discussion of that wish is the week's most important teaching about the difference between the model's respect and the therapist's need to be useful.

11.9 Six errors in narrative work

1. **The problem that never costs anything.** Rushing to unique outcomes without mapping the problem's influence, producing a session that feels light and leaves the family unheard. Fix: fifteen minutes on the left column, minimum.
2. **The therapist's vocabulary.** "Externalising", "re-authoring", "unique outcome", "absent but implicit" used in the room. Fix: the family's words only; the technical vocabulary is for supervision.
3. **The clever reframe.** Turning the family's story into something more interesting to the therapist. Fix: the question rather than the formulation, and a supervisor who asks who the session was for.
4. **The individual in a family session.** Externalising with one member while the others watch, so that the problem's relational effects are never mapped. Fix: name with everyone, map with everyone, and ask each member for their name for the problem.
5. **The story without a plan.** A beautiful preferred story with no change in the household's Tuesday. Fix: the final questions are practical — who, when, what, and how will we know?
6. **Risk mistaken for a story.** A safeguarding concern treated as a dominant narrative. Fix: the same screen as Week 10, performed before the first narrative question, and the referral process honoured.

11.9a What the model does with a diagnosis

A specific application, because it is where the model's politics become clinical practice.

The situation. A family referred with a child's diagnosis — autism, ADHD, a genetic syndrome, a psychiatric label — and a file that has become the family's story. The parents introduce themselves through the diagnosis: "we are the parents of a child with…".

What the model does not do. It does not dispute the diagnosis, minimise the impairment, or suggest that the family's difficulties are a matter of perspective. That would be both clinically wrong and cruel, and it is not what the model proposes.

What the model does. It separates the diagnosis from the description of the child and the family, and it asks about the diagnosis's influence: "what has the diagnosis made possible? What has it made difficult? Who in the family has been recruited by it? What does it not describe about your son?" The last question is the one that opens the work, and parents answer it readily and with relief: the diagnosis does not describe that he is funny, that he is kind to his sister, that he notices when his mother is sad.

The two-column mapping, for a diagnosis. Left: what the label has done — appointments, explanations to relatives, an identity for the parents, a way of understanding the child's behaviour, sometimes a route to services. Right: what the family has done that the label does not account for — the adaptations they invented, the teacher they persuaded, the night they managed a crisis, the language they developed for their child's states.

The preferred story. Not "our child is not autistic", which would be false and would be the therapist's imposition, but "we are a family who have learned our son's language, and the label is one of the tools we use, not the whole of him". Families produce versions of this sentence readily, and the sentence changes the household's relationship to the file.

The witness. Where a diagnosis has organised a family's life, an outsider witness who is not a professional is powerful: a relative, a family friend, another parent. The witness's four-part response (striking, image, what it says about the person, why it matters to me) produces a public description that competes with the clinical one, and it is usually the first time the family have heard their competence described by someone who is not paid to do it.

The boundary. Where the diagnosis requires medical management, the model assists and does not substitute. The narrative work runs alongside the paediatric or psychiatric care, with the family's consent and often with the consultant's knowledge, and the therapist is explicit that the model addresses meaning and relationship and not the condition itself.

11.10 Culture, faith and the model's critique

Where the model travels well — better, arguably, than any other in this book. Its refusal to diagnose, its insistence that the family are the authors of their own description, and its interest in the effects of institutional labels are all congenial to communities that have been described by institutions. Narrative practice originated partly in work with Aboriginal communities in Australia and has been developed in community and collective settings, in work with refugees, and in settings where the dominant story about a group is itself the problem. The techniques are also cheap: a conversation, a notebook, a letter.

Where it strains. First, the model's linguistic register. The questions are simple, and the theory is sophisticated, and the theory has a way of migrating into the questions in the hands of an enthusiastic trainee; a family who are asked when "the problem first recruited them into its service" will conclude that the therapist is strange. Second, the model's

emphasis on the individual's authorship can sit awkwardly with collective decision-making; in a family where the elders' story is the family's story, the therapist's invitation to the young wife to re-author her own life may be an invitation to a conflict she cannot win. Third, the model can under-attend to material reality: a family whose problem is a leaking roof and a landlord's indifference does not need a new story, and the therapist who offers one is committing the error that the model's critics name most often.

Faith. The model is unusually at home with religious language, because religious communities are story-bearing communities: scripture, testimony, pilgrimage and ritual are narrative practices. A family's story of a difficult season may be framed within a larger story of providence, and the therapist who treats that frame with respect will find it a resource; the therapist who treats it as a defence will lose the family. The model's caution is also relevant: the therapist must not supply a theological story of their own and call it the family's preferred narrative.

> **DEBATE**
> "Narrative therapy is the model most likely to be loved by therapists and least likely to be tested by researchers. Is that a coincidence?" Argue both sides.

11.10a The two strongest critiques, and the model's answers

Critique one: it can talk a family out of a material problem. The classic objection. A family living in overcrowded housing, with a child unable to sleep and parents unable to work, can be offered an externalising conversation about "the crowding", and the conversation is both true and useless. The model's focus on meaning can produce a therapy that adjusts a family to an injustice instead of naming it.

The model's answer. Narrative practice has an explicit tradition of working with material conditions and with the effects of poverty, and its community work is often advocacy; the model's techniques can be used to name an injustice as an external problem with an address ("the landlord", "the housing office") and to organise action against it. The model does not require the therapist to treat material problems as stories.

The fair verdict. The answer is adequate, and the risk is real and common. The safeguard is procedural: the formulation must include the material conditions, and the therapist must ask whether the family's problem can be changed by a conversation or requires an action, a referral or an advocate.

Critique two: the evidence is thin relative to the model's influence. Narrative therapy is widely taught, widely admired and comparatively little tested. The trials that exist are mostly small, often practitioner-led, and concentrated in a few presentations (depression in adults, some child and family work, community applications). There is no large independent trial of narrative family therapy comparable to the EFT literature.

The model's answer. The model's evidence base is growing; its components (externalising, unique outcomes, the decentred therapist) are congruent with common factors and with the broader evidence on client-directed work; and the model's refusal of manualisation is

a principled position rather than an evasion, since manualisation would fix the very descriptions the model wants to leave open.

The fair verdict. The position is principled and it does not discharge the obligation. A model that is widely practised with families should be tested, and the honest practitioner will describe narrative work as "promising, with a small evidence base" and will contribute to outcome data rather than citing the model's reputation.

11.11 Evidence note: promising, thin, and honest about it

What exists. Randomised trials of narrative therapy exist for adult depression and for some anxiety presentations, with small to moderate effects; there is qualitative and small-scale evidence for family and child applications; and there is a substantial practice literature from community and collective settings. Trials of narrative family therapy are few, small, and mostly single-site.

What the model's components contribute. Externalising, unique outcome enquiry and the decentred therapist's stance are all consistent with the common-factors literature (Week 11's matrix and MFT 728), and the model is probably effective partly through mechanisms it shares with every other model in this book: alliance, hope, a coherent explanation, and a therapist who is genuinely interested.

What the evidence does not show. That narrative therapy outperforms other family approaches; that its techniques are separable from the therapist's manner; that it is safe or sufficient where risk, violence or acute psychiatric illness is present; and how it performs across cultural settings, where its practice literature is far richer than its trial literature.

The matrix cell. "Small randomised support for adult depression and some anxiety presentations; family evidence is small-scale and largely qualitative; components congruent with common factors; strong practice and community literature; not indicated as a sole approach where risk or acute illness is present."

The practitioner's claim. "I use narrative work where a family has been described by an institution or by a diagnosis, because it restores authorship quickly and respectfully; I map the problem's influence before I look for preferred stories; I keep the material conditions in the formulation; and I measure outcome, because the model's evidence is thin." That sentence is defensible in a viva and honest in a case discussion.

11.12 Key terms

Term	Definition
Dominant story	The authoritative account of a family's life, often authored by an institution
Thin description	A label; a description without events, intentions or context
Thick description	A story with particulars: what happened, who did what, what it meant
Externalising	Separating the problem from the person and giving it a name and an influence
Mapping the influence	Two enquiries: the problem's influence on the family, and the family's on the problem

Term	Definition
Unique outcome	A moment that falls outside the dominant story
Re-authoring	Building a preferred story from unique outcomes
Re-membering	Deliberately including significant figures, living or dead, in the construction of identity
Definitional ceremony	A structured occasion in which an audience witnesses and reflects on the preferred story
Outsider witness	A chosen audience member who responds in a fixed four-part sequence
Absent but implicit	Reading a value from a gap or loss
Decentred but influential	The therapist influences the conversation and does not author the story

11.13 Self-test

1. State the model's foundational principle and explain why it is strategic rather than merely kind.
2. Distinguish a thin from a thick description, with an example.
3. Give the five prompts used to map the problem's influence.
4. What are the two columns, and why must the left one be completed first?
5. What is a unique outcome, and what three questions thicken it?
6. What is the four-part outsider witness response?
7. State the two strongest critiques and the fairest verdict on each.
8. What does the model do, and not do, with a child's diagnosis?

Answers. 1. "The person is not the problem; the problem is the problem." It is strategic because a person identified with a problem must defend themselves, while a person in a relationship with a problem can negotiate it. 2. Thin: "negligent mother". Thick: a story of a woman who lost her husband, took an early shift, sleeps through alarms, and once swallowed her pride to ask her sister for help. 3. What it makes you do; what it prevents; what it makes you think about yourself; what it does to your relationships; what it has cost. 4. The problem's influence on the family, and the family's influence on the problem; the left column first because the family must be heard in full before any agency is claimed, and because a family who have not said how bad it is will not build a story. 5. A moment outside the dominant story; thickened by asking what the person did, what it required of them, and what it says about what they value. 6. What struck me; what image stayed with me; what this says about the person; why it matters to me. 7. Critique one — it can talk a family out of a material problem; fairest verdict, the risk is real and the safeguard is to keep material conditions in the formulation and to act where action is required. Critique two — the evidence is thin relative to its influence; fairest verdict, principled refusal of manualisation does not discharge the obligation to test and to measure. 8. It does not dispute the diagnosis or minimise impairment; it separates the diagnosis from the description of the child, maps the diagnosis's influence in both directions, and asks what the label does not describe — while medical management continues alongside.

11.14 Reflective task

Write the description of yourself that a school, an employer or a clinic would produce if it observed you for a month. Then write the description your closest friend would produce. Put them side by side and mark which sentences are thin and which are thick, and which description you would want a stranger to read. The exercise is the model's politics applied to the practitioner, and it usually produces a sharp sense of why the referral's language matters.

11.15 Readings for this week

- **White & Epston (1990)**, Narrative Means to Therapeutic Ends, the founding text, read the chapters on externalising and unique outcomes with a transcript beside you.
- **White (2007)**, Maps of Narrative Practice, the mature statement, especially the chapters on re-authoring and re-membering.
- **Morgan (2000)**, What is Narrative Therapy?, the clearest short introduction, suitable for clients as well as students.
- **Freedman & Combs (1996)**, Narrative Therapy, the American statement, useful for the conversational structure.
- **Denborough (2008)**, Collective Narrative Practice, for community and collective applications.
- **Lopes et al. (2014)** and the adult depression trials, for the outcome evidence, read critically for sample size and independence.

Read Maps of Narrative Practice slowly, with a notebook, and copy out the questions rather than the theory; the questions are the model. For the essay, distinguish the model's politics from its evidence and say what each can and cannot support. The reading-log question this week is: "whose description of this family is currently the most powerful, and what would change if mine became as powerful?"

Next: Week 12 — Integration without incoherence

Week 12 — Integration without incoherence

The question. How do you combine models so the client is not paying for your confusion?

Three legitimate forms of integration:

1. **Assimilative integration.** A home model, enriched by techniques from elsewhere. Most clinicians work this way. The home model keeps the formulation coherent.
2. **Sequential integration.** Different models at different phases — for example structural work to establish safety and hierarchy, then EFT for the couple's bond, then solution-focused work to consolidate.
3. **Theoretically integrated models.** Purpose-built blends (integrative behavioural couples therapy; attachment-based family therapy), coherent because they were designed together.

Rules for staying coherent.

Rule	Why
One formulation, one primary target	Prevents scattergun technique
Name the model out loud in supervision	Makes your reasoning auditable
Change model for a reason you can state	"Because the conflict escalates when we go near feelings"
Do not mix incompatible theories of change in one move	Reframing and behavioural contracting pull in different directions
Review at fixed intervals	Integration fails by drifting, not by decision

Trap to avoid: technique salad. Symptom-tracking from CBT, a miracle question, a family sculpture and an externalising conversation in one session does not make you integrative; it makes you unpredictable, and the family cannot tell what is expected of them.

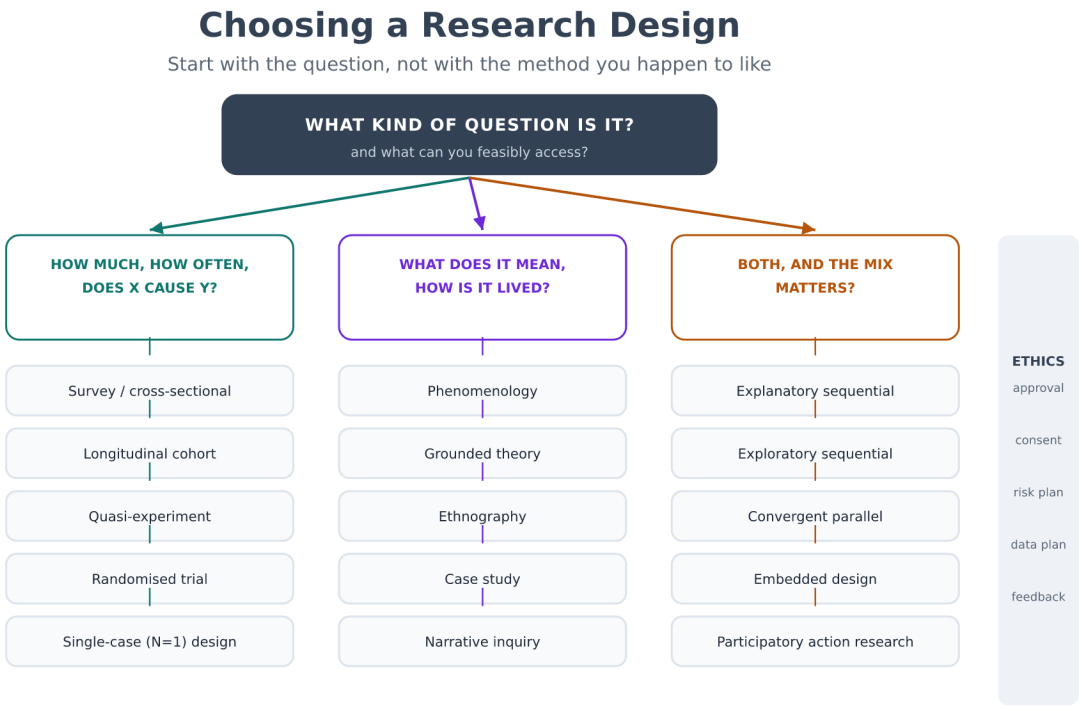


Figure 12.1 — Choosing a model: the decision tree a clinician actually uses in the first two sessions

> **ASSIGNMENT (30%)**
> One disguised case; two models; one formulation each; a reasoned choice of sequence; a predicted risk of the combination; a plan for how you would know it is not working.

12.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The problem	Why every practitioner integrates and why most do it badly	Name their own home model, honestly
10–25	Three forms	Assimilative, sequential, theoretically integrated, with an example each	Classify four published treatments
25–40	Coherence	The five rules, applied to a live case	Apply the rules to a case they are holding
40–55	Choosing	The decision tree: risk, target, phase, evidence, fit	Place their case on the tree
55–70	The handover	What changes when you change model mid-therapy, and how to say it	Write the handover sentence
70–80	Technique salad	A recording of a session that mixes four models badly	Diagnose it in five minutes
80–90	Assessment briefing	The 30% case study, its rubric, and what distinction looks like	Sketch the structure of their answer

The teaching decision that makes the week work. This is the week in which the course turns from description to judgement, and the lecture should therefore be built around a single case that the class has already met — the Adeyemis — and a single question: which model, in which order, for this family, and why? The lecturer should write the answer on the board as a sequence and then attack it: where is the evidence, what is the risk of the second step, what would make you change the plan? The class should leave with the experience of having built an integration and having had it examined, which is exactly what the assessment requires.

The sentence the lecture must land. "Integration is a sequence of justified decisions, not a collection of techniques." A student who can state their home model, name the conditions under which they would leave it, and describe how they would know the decision was wrong has understood the week.

12.2 Why every practitioner integrates, and what goes wrong

The fact. No practitioner delivers a pure model after the first year of practice. The clinic's population does not match the model's trials; the family presents three problems when the model addresses one; the therapist's training was in two traditions; and the model's manual does not cover the case in front of them. Integration is therefore not a sophisticated option but an inevitability, and the choice is between doing it deliberately and doing it by drift.

Drift, described. A therapist begins with a structural formulation, uses an EFT enactment in session four because the couple's affect is high, introduces a behavioural contract in session six because the enactment did not produce change, externalises "the silence" in session eight because the couple are stuck, and ends with a solution-focused scaling in session ten. Each decision was locally reasonable; the therapy as a whole has no

formulation, no target, and no way for the family or the supervisor to know what is being attempted. The family experience this as a therapist who is trying things.

Coherence, described. The same therapist, with the same repertoire, works from one formulation with one primary target, and uses a technique from another model at a point where the formulation predicts it will help — and can say, in supervision, why. "The formulation is a pursue–withdraw cycle with a structural component: the father's absence leaves the mother enmeshed with the daughter. My primary target is the couple's cycle. I used a structural move in session three because the boundary between mother and daughter was blocking the couple work, and I returned to the cycle in session four." That paragraph is integration.

The four failure modes.

Technique salad. Many techniques, no formulation; the family cannot predict the therapist and the supervisor cannot audit the reasoning.

Theory soup. A formulation that borrows concepts from four models without reconciling them: a family described as having a disorganised attachment, a triangulated child, an insufficiently differentiated father and a problem-saturated story, with no statement of which of these is doing the explanatory work.

The serial monogamist. A therapist who changes home model every two years, each time convinced that the new one is the answer, and who has therefore never learned any of them deeply enough to be flexible within it.

The purist. A therapist who delivers one model faithfully to families for whom it is not indicated, because fidelity has become an identity. This is the rarest failure and the most damaging, because it is usually accompanied by a formulation that blames the family for not fitting.

The common thread. All four fail in the same place: the therapist cannot state, in one paragraph, what is wrong, what is being targeted, what is being done about it, and how they will know whether it worked. The remedy is procedural rather than theoretical, which is why this week is practical.

12.2a A formulation that travels, and one that does not

Two formulations of the same family, written for a case file, to show what coherence looks like on paper.

The formulation that does not travel.

"The Okonkwo family presents with a nine-year-old's defiance, a mother who is overwhelmed and possibly depressed, a father who is emotionally disengaged, and a family whose boundaries are enmeshed in the mother–son dyad and disengaged in the marital subsystem. There is unresolved grief following the paternal grandmother's death, which the family have not processed. The child's behaviour serves a triangulating function in the marital conflict, and the family's communication is characterised by double binds. The treatment plan is to work on boundaries, process the grief, improve communication and address the mother's depression."

What is wrong with it. Four models' vocabularies in five sentences (structural boundaries, Bowenian differentiation, psychodynamic grief, MRI double binds, behavioural communication), no statement of which concept is doing the explanatory work, four targets and no primary one, and a treatment plan that is a list. A supervisor reading it cannot tell what the therapist will do in session three, and the therapist cannot either.

The formulation that travels.

"Formulation (structural, primary): this is a family with a weak executive subsystem. Chuka works nights, so the parents do not act together; Ada manages alone, and her management has become a coalition with Ebuka that excludes his father. The grandmother who regulated the household died in 2023, and no arrangement has replaced her. The defiance is the visible edge of the coalition: Ebuka tests Ada and she cannot hold a line without support, and Chuka's interventions are either absent or too late.

Primary target: the parental subsystem, so that the two of them can act together on one rule, this month.

Secondary observations, not targets: Ada's low mood (screen for depression, refer if indicated); the family's grief (a bereavement service is available and Ada has been offered it); Ebuka's school (a behaviour plan is in place and is working).

Evidence of progress at six sessions: the parents give the same answer to the same request on three consecutive occasions; Ada reports fewer evenings alone with Ebuka; Chuka reports one joint decision per week.

Risk to the plan: Chuka's nights may make the joint work impossible in person; if so, the plan becomes a telephone-based executive meeting at a fixed time, and the formulation is unchanged."

What makes it travel. One vocabulary, one primary target, secondary observations explicitly not targeted, a measurable criterion, and an anticipated obstacle with a designed response. A supervisor can read it and know what will happen in session three; the therapist can read it in a difficult session and remember what they are doing.

The lesson for the assessment. The 30% case study is marked on this distinction. A student who produces the first paragraph, however impressive its vocabulary, has not integrated anything; a student who produces the second has done the week's work, in about three hundred words.

12.3 The three forms, described properly

Assimilative integration. The therapist has a home model — the one in which they are trained, which organises their formulation and their sense of what a session is for — and incorporates techniques, concepts or stances from other models into it. The home model absorbs the imports, and the formulation stays coherent because the home model's logic governs when and why an import is used.

An example. A structural therapist working with the Okonkwos. The formulation is structural: a weak parental subsystem, an enmeshed mother–son dyad, and a

grandmother whose death removed the family's regulator. The therapist borrows a behavioural exchange from Week 8 — not as a behavioural treatment but as a structural intervention, to give the parents a joint task that enacts their executive function. The borrowed technique is subordinate to the structural aim, and the therapist can say so.

What makes it legitimate. The import has a specified function in the home model's terms; it is used at a point the formulation predicts; and it is abandoned if it does not serve the aim.

What makes it dishonest. The import becomes the therapist's real model while the formulation continues to use the home model's vocabulary. A therapist who formulates structurally and works behaviourally is practising behavioural therapy under a structural description, and the description will eventually mislead them and their supervisor.

Sequential integration. Different models are used at different phases of the work, with an explicit rationale for the sequence. The phases are usually determined by risk and by the family's capacity: safety, then structure, then affect, then consolidation.

A worked sequence for a family with violence and a child's symptom. Phase one, safety and stabilisation (individual work, safety planning, a group for the person who has used violence — MFT 719). Phase two, structural family work to re-establish an executive subsystem and a boundary around the children's role in the parents' conflict. Phase three, EFT for the couple's bond, once coercion is absent and the couple can tolerate affect. Phase four, solution-focused consolidation, with the ending criterion the family names.

What makes it legitimate. Each phase has an entry criterion (what must be true before it begins), an exit criterion (what indicates it is done), and a stated reason why the next model rather than the same one. The sequence is written down and reviewed.

What makes it dishonest. The sequence is retrospective: the therapist changed models because the last one was failing, and describes the change afterwards as a plan.

Theoretically integrated models. Purpose-built blends, designed as one treatment rather than assembled by a practitioner. IBCT (Week 8) is one: acceptance and change were designed together, with a shared formulation and a shared theory of the mechanism. Attachment-based family therapy is another; so is emotionally focused family therapy; so is multidimensional family therapy for adolescent substance misuse. These models are coherent by construction, they have manuals, and several have trial evidence.

The practitioner's relationship to them. They are preferable to home-made integration where the presentation matches: a therapist who works with adolescent substance misuse should use a purpose-built model with evidence rather than assembling one. The limitation is coverage: the built models exist for a handful of presentations, and most families in a general clinic do not match.

The fourth form, which the textbook omits. Integration by setting. A therapist working in a public clinic integrates not only models but constraints: six sessions, no team, a family who travel two hours, no interpreter. The model that survives these constraints is not the model in the manual, and the practitioner's honesty about the

adaptation is part of the integration. This form is the one most students will actually practise, and the course should name it.

12.3a The five rules, applied

Rule one — one formulation, one primary target. The formulation may be complex; the primary target is singular. "The primary target is the couple's pursue–withdraw cycle. The child's school refusal is being addressed by the school and by a graded return plan, and I am not targeting it in the family sessions."

Why it matters. A therapy with three primary targets has no way to evaluate itself, and the family experience it as an absence of direction.

How it fails. The therapist writes a formulation with four targets and calls them "interrelated", which is true and unhelpful.

Rule two — name the model out loud in supervision. "In sessions one to four I worked structurally; in session five I switched to EFT because the escalation when we touched the father's absence was beyond what a structural move could hold." The naming makes the reasoning auditable, and audit is the only defence against drift.

How it fails. The therapist describes what they did without naming the model, so the supervisor cannot see that two incompatible theories of change are operating in the same session.

Rule three — change model for a reason you can state. The reason must be about the case, not about the therapist. "Because the conflict escalates when we go near feelings" is a case reason. "Because I am better at EFT" is a therapist reason, and it may still be a legitimate factor, but it must be named as such.

Rule four — do not mix incompatible theories of change in one move. This is the rule students find hardest, because it forbids the most tempting improvisations. A reframe ("your son's defiance is holding the family together") and a behavioural contract ("three items each, reviewed weekly") in the same session give the family two incompatible instructions: one says the problem has a function and should be understood; the other says the problem is a behaviour to be changed. The family will comply with the easier one and the therapy will lose the harder.

The repair. Sequence them, with a stated rationale: "we have understood what the defiance is doing; now we will change one small piece of it and watch what happens to the family." The two moves are now in a sequence with a rationale, which is integration rather than salad.

Rule five — review at fixed intervals. Every sixth session, a review that asks four questions: is the primary target still the right one; is the model still the right one; what has changed and what has not; and what would make me change the plan? The review is written, and it is the audit trail.

How it fails. The review happens only when the therapy is failing, which means the therapy has no monitoring and the review is a post-mortem.

12.4 Choosing: the decision tree

The lecturer's tree, in the order a clinician actually uses it in the first two sessions.

Step one — is there a risk that decides the treatment? Violence, abuse, self-harm, a child not safe, an acute psychiatric episode, an addiction that makes talking useless. If yes, the treatment plan begins with the risk, and the model is chosen accordingly: safety work, statutory process, psychiatric liaison, detoxification. No model in this book is indicated as a first response to these.

Step two — what is the family's primary target, in their language? Not the referral's target: the family's. A family referred for a child's behaviour who want their marriage back are telling the therapist something about the target.

Step three — what is the best-evidenced model for that target? EFT for relational distress without coercion; behavioural couple therapy for a structured, time-limited couple piece; structural family therapy for child and adolescent behaviour in a family with an executive problem; solution-focused work where the goal is nameable and brief; narrative where a family has been described by an institution. The evidence narrows the field to two or three plausible models.

Step four — which of those can this family actually do? Attendance, language, work patterns, childcare, literacy, the number of sessions available, whether the family will accept a team. A model the family cannot attend is not indicated, whatever its evidence.

Step five — which can I deliver with fidelity? Training, supervision, the setting's constraints, the therapist's own capacities. A therapist who has not been trained in EFT should not begin an EFT case with a complex couple; the honest answer is a referral, a different model, or supervised practice.

Step six — what will tell me it is working? A measure, a review date, and a criterion the family helped set. Without this, step five's answer cannot be tested, and the therapy becomes a matter of conviction.

The tree's use in the assessment. The 30% case study asks for exactly this sequence: two models formulated, one chosen, the choice reasoned, the risk predicted, the failure criterion named. A student who follows the tree will produce a defensible answer; a student who begins with the model they like best will produce a list of techniques.

12.4a A worked integration: the Adeyemis, five phases

The course's recurring family, integrated deliberately, with the reasoning written out.

The case in one paragraph. Amara (14) has refused school for five weeks; her mother Funke (41) has taken leave; her father Tunde (46) works in Lagos and attends by telephone; her brother Segun (11) is silent and manages the household's information. There is no violence. There is a bullying incident at school that the parents do not know about, a paternal grandmother who died in 2023, and a family economy that depends on Tunde's contract.

Phase one — assessment and risk (sessions 1–2). Structural and systemic assessment together with the risk screen. The formulation: a family in transition, with a

developmental stage missed (MFT 703), a weak executive subsystem because of the father's absence, an enmeshed mother–daughter dyad, and a school problem that the family's organisation cannot address. Primary target: the school refusal, in its family and school context.

Phase two — the school (sessions 2–4). The model choice here is not a family therapy model at all: a behavioural graded-return plan, negotiated with the school, with a named adult and a seating change. The reason for the sequence: no family work will hold while a child is being bullied, and the family's anxiety is being organised by a real external danger. A purist family therapist would begin with the family system; the integrated practitioner begins with the school, because the formulation says the school is part of the system and the risk is there.

Phase three — structure (sessions 4–8). Structural family therapy, chosen because the family's organisation is the second component of the formulation: enactment of a parental subsystem that can act together, a boundary around the mother–daughter dyad, and the father's authority re-established by telephone and by a monthly visit. The technique of choice is enactment, and the therapist works in the session rather than through advice.

Phase four — the couple's bond (sessions 9–14). EFT, chosen when the structural work has produced enough organisation for the couple to tolerate affect, and when Funke and Tunde's cycle — she pursues, he withdraws — has become the family's main obstacle. The phase's entry criterion is explicit: a fortnight with no escalation, the father present in person for two sessions, no violence. The work follows Week 9's stages, abbreviated, with one softening event in session twelve.

Phase five — consolidation (sessions 15–18). Solution-focused, chosen because the remaining task is to consolidate the family's own solutions rather than to produce change: the ending criterion, the exceptions that now exist, the plan for the next crisis, and a review at three months.

The risks of this integration, stated. First, the sequence is long: eighteen sessions is more than the clinic's budget, and the therapist must be prepared to compress phases three to five if the funded episode ends. Second, the transition from structural to EFT is the fragile point: a couple who have been organised by a parental task may experience the emotional work as a regression, and the therapist must frame it as the next stage rather than as a new therapy. Third, the school phase depends on the school's cooperation, which may fail, and the formulation must then include the school's refusal as part of the system. Fourth, the father's absence is a structural fact that no amount of EFT can change: the couple work must be adapted to the medium, and the therapist must say so rather than pretending a telephone is a room.

How the therapist would know it is not working. At the review in session eight: no change in attendance; no change in the parents' joint action; the couple's cycle unchanged and the escalation worse. At that point the plan changes — a different primary target, a different model, or a referral — and the change is recorded with its reason, which is the difference between integration and drift.

12.5 The handover: how to change model without losing the family

The practical skill the week exists to teach. A change of model is a change of what the therapist asks, how the session is structured, and what the family is expected to do; unexplained, it is experienced as inconsistency or as a verdict that the first approach failed.

The handover sentence, in four parts. Name what has been done; name what it has achieved; name what it has not achieved; name what will be different from now on.

Worked. "For the last six sessions we have been working on how the four of you handle the evenings — who decides, who backs whom up, and what happens when Ebuka tests it. That has changed: the two of you are now giving the same answer, and the shouting has dropped. What has not changed is the tension between the two of you, and I think that is now the thing that will pull the old pattern back. So for the next few sessions I will work with the two of you alone, on that, and the children will come back at the end."

What makes a handover work. It is given before the change, not after; it credits the family for what has been achieved; it is honest about the limitation; and it specifies who will be present and for how long, which families need in order to tolerate a change.

What makes it fail. Announcing a new technique without framing it ("today we are going to try something different"); changing the session's composition without explanation (asking the children to leave); and implying that the first phase failed ("we need to start again"), which invites the family to conclude that the therapy is not working.

The recorded variant. For a family who cannot attend a transition session, a written handover of four paragraphs follows the same structure, and it is read aloud at the start of the next session.

The supervision note. The handover double-functions as the audit record: a supervisor reading it can see the model change, its reason, and its expected effect, which is exactly what Rule two requires.

12.5a The first two sessions of an integrated therapy

The plan is written before the work; the first two sessions decide whether it survives. A structure that serves every form of integration.

Session one — assessment and the family's own account. The therapist takes the family's account without a model's vocabulary, notes the referral's language and the family's language side by side, screens for risk, and asks the two questions that determine the target: what would you want different, and what have you already tried? The session ends with a frame — how many sessions, who attends, what will happen next — and no technique.

Between sessions one and two — the written formulation. One page, in the structure of 12.2a: the formulation in one model's terms; the primary target; the secondary observations that are not targets; the criterion of progress; the risk to the plan. Written before session two, not after session four.

Session two — the formulation, offered and tested. "Here is what I think is happening, and here is what I propose we work on first. I may be wrong; tell me what fits and what does not." The family's corrections are recorded, and the formulation is revised in front of them. This is the moment at which the therapy becomes collaborative, and it is also the moment at which the family learn that the therapist has a plan — which is what makes the later handovers tolerable.

Why this order rather than the reverse. A therapist who begins with technique and formulates later has already taught the family what the therapy is, and the formulation then arrives as a reinterpretation of work already done. A therapist who formulates after one session and offers it in the second has a plan the family can join, correct, and hold the therapist to.

The exception. Where risk is present, session one ends with a safety plan and session two begins with it; the formulation waits until the safety work is in place. The order of the two sessions is a clinical judgement, and the judgement must be recorded.

12.6 Case: the same family, integrated badly

The Adeyemis again, with the integration done by drift, so that the errors can be named.

Sessions one to three, no formulation. The therapist takes a history, asks about the school refusal, and gives advice about routines. The family comply politely and nothing changes.

Session four, a behavioural contract. A morning routine is designed — alarms, charts, rewards — and it collapses within a week because Amara is being bullied at school and the morning is the moment of terror, not of laziness. The therapist concludes that the family is not motivated.

Session five, a structural enactment. The therapist asks the parents to act together in the room. The father is on speaker. The enactment produces an argument about the telephone, and the therapist, anxious, moves on.

Session six, an EFT-style question. "When you hear your wife say that, what happens in you?" The father answers in monosyllables; the mother cries; the therapist deepens; the father ends the call. The therapist concludes that the model needs both partners present.

Session seven, externalising. The therapist asks the family to name "the worry". They do, and the conversation is the best of the seven, and the therapist does not follow it up because the next week brings a school meeting that consumes the session.

Session eight, a scaling question. "Where are you, nought to ten?" The family answer. The therapist offers a task. The therapy ends at session ten by the family's non-attendance.

The diagnosis, in the model's own terms. Six techniques from five models; no formulation; no primary target; no review; no handover; and no hypothesis about the bullying, which was mentioned in session two and never followed. The family's experience was of a therapist who tried things, and their conclusion — that therapy is a series of conversations that do not add up — is entirely reasonable.

What a supervisor would have caught. At week two: "what is your formulation, and what are you targeting first?" At week four: "the contract failed; what does the failure tell you?" At week five: "you changed model; why, and what did you tell the family?" At week seven: "that was good work; why did you not continue it?" None of these questions requires expertise; all of them require the therapist to have written the plan down.

The counterfactual. The integrated plan of 12.4a, delivered in the same number of sessions, would have begun with the school, used the structural work to organise the parents, and used the couple's cycle as the primary target in the second half. The difference is not talent; it is the presence of a formulation and a sequence.

12.6a When to stop integrating and refer

The professional decision that integration makes easy to postpone: some families need something the integrative therapist cannot provide, and the honest response is a referral rather than another model.

The four signals.

Signal one — a risk that needs a specialist service. Violence requiring a perpetrator programme; a child requiring a safeguarding process; an addiction requiring detoxification; a psychiatric crisis. The therapist remains involved where appropriate and is not the lead.

Signal two — a presentation outside the therapist's training. A couple presenting with a sexual difficulty, a family with a member with an eating disorder in acute phase, a family requiring a court report. Integration is not a licence to practise beyond competence, and the assessment rubric rewards knowing this.

Signal three — no change after a well-formulated, well-delivered plan. Two phases, two formulations, a review at each, and no change. The next step is consultation, not a third model. The most common professional failure in an integrative practitioner's career is the third, fourth and fifth model applied to a family who needed a different therapist or a different kind of help.

Signal four — the therapist's own reaction. Boredom, dread, resentment, or an unusual wish to be admired by a particular family. These are data (MFT 735, supervision), and they are frequently the earliest signal that the work has gone wrong.

How to make a referral that keeps the family. Name the reason precisely and without self-blame; name what has been achieved, so the family do not experience the referral as a verdict; make the referral warm, with a telephone call and a joint session where possible; and follow up once. The family's experience of being passed on is determined almost entirely by how the referral is done, not by the fact of it.

The sentence. "I am not the right person for this piece of work, and I want to say why, and what I think you need." It is the most useful sentence in this week, and the hardest to say.

12.7 Annotated transcript: the review session

Ten turns from a sixth-session review with the Okonkwos, where the therapist has to decide whether to continue the structural work or change target — the moment when integration is either done or drifted into.

> **T1 Therapist:** This is our sixth meeting, and I always stop at six and ask four questions. The first is whether we are working on the right thing. When you started, you said the thing you wanted different was the evenings. Is that still it?
> **T2 Ada:** The evenings are better. It is the weekends that are bad now.
> **T3 Therapist:** So the target may have moved. Tell me about a weekend.
> **T4 Ada:** Saturday he is home all day. And he does nothing with Ebuka, and then by six they are shouting.
> **T5 Therapist:** (to Chuka) What is your view of the weekend?
> **T6 Chuka:** I work all week. Saturday is the only day I have.
> **T7 Therapist:** So the evenings improved because you had a clear arrangement, and the weekends have no arrangement at all. Is that fair? (both nod) Then I want to change one thing about the plan. The target is no longer the evenings; it is the weekend, and the two of you are the ones who have to decide what it looks like. That is the same work we have been doing, on a different day.
> **T8 Ada:** So we are not starting again.
> **T9 Therapist:** No. Same work, new day. And I want to add something: I think the two of you disagree about what a father should do on a Saturday, and we have never asked that question. Next week I will ask it.

> **T10 Chuka:** (pause) Nobody has ever asked me that.

Annotations. T1 is the review's first question, and the framing matters: the therapist announces the review, states its purpose, and asks the family's view before offering their own. The review is a procedure, not a conversation about how things are going.

T2–T4 is the information the review is for: the target has moved. The therapist does not defend the original target or treat the change as a failure; she asks for the detail, which produces the weekend.

T5–T6 brings in the quiet partner with a question about his view rather than his behaviour, which is the structural move that keeps him in the therapy.

T7 is the integration decision, made aloud: the target changes, the model does not, and the reason is given in the family's language ("the same work, on a different day"). This is Rule three demonstrated: the change is stated with a reason the family can hear.

T8 is the family's question, and it is the one every handover must answer: are we starting again? T9's answer — "same work, new day" — is the sentence that keeps the therapy's continuity intact.

T9 also adds a second model's concept, carefully subordinated: the disagreement about what a father should do is a relationship standard (Week 8), imported into a structural

therapy as a question to be asked, not as a behavioural programme. The import is announced in advance, which is what makes it legitimate.

T10 is why the import was worth making: the father has never been asked what a father should do, and the question has mobilised him. A therapist who had followed the manual would have missed it; a therapist who improvised would have missed the framing that made it safe.

What the trainee should notice. The whole exchange is eight minutes, it changes the plan, it keeps the family, and it produces the therapy's best material. Integration of this kind is not a theoretical achievement; it is a discipline of reviews and handovers, performed in ordinary language.

12.7a Two integrations that should not have been attempted

The incompatible pair. A therapist working with a couple in which one partner had disclosed a recent affair combined a behavioural contract (weekly transparency, phone access, a written schedule) with an EFT deepening of the betrayed partner's attachment fear in the same session. The two moves pull in opposite directions: the contract addresses the betrayal as a trust-building task, and the EFT work asks the betrayed partner to express vulnerability to the person who has just wounded them. The session ended with the betrayed partner in tears and the unfaithful partner describing the contract as surveillance. The error: two theories of change in one move (Rule four). What would have worked: a sequence, with the affair work first (a structured disclosure and a plan, informed by the affair literature), and the attachment work later, when the disclosure had been absorbed.

The integration that concealed a risk. A therapist working with a family referred for a child's behaviour combined solution-focused work with a narrative externalising of "the tension", and the sessions were warm and productive. In session nine the mother disclosed that the father had been physically violent. The externalising had made "the tension" an entity that neither parent owned, and the therapy had spent eight sessions building a description in which nobody was responsible for anything. The error: the model's non-normativity, used twice over, had produced a description with no agent. What would have worked: the screen in session one, and a formulation in which the violence was named as a person's behaviour rather than as the family's atmosphere.

The general lesson. Integration fails most damagingly not when it is incoherent but when it is coherent around the wrong formulation. A therapy can be internally consistent, well sequenced and warm, and still be organised around a description that protects the person causing harm — which is why the review must include a question that no model asks: what is not in this formulation, and who benefits from its absence?

12.8 Laboratory: the integration audit

Sixty minutes, in threes, using a case each student is currently holding or has observed.

Task one — the paragraph (fifteen minutes). Each student writes one paragraph, in five sentences: the formulation; the primary target; the model being used and why; the model that was considered and rejected and why; and the evidence that will show whether

it is working. The paragraph is read aloud and the other two students mark it against the five sentences, naming any that are missing or vague.

Task two — the drift map (twenty minutes). Using a case of more than four sessions, the student writes the sequence of models actually used, session by session, and marks each transition with a reason or with a blank. The blanks are the drift, and the exercise's point is that most students find two or three.

Task three — the handover (twenty-five minutes). The student drafts the four-part handover sentence for the next transition they anticipate, reads it aloud, and the group rewrites it until it names what has been achieved, what has not, and what will change. The criterion: could the family repeat it back accurately?

Marking rubric for the 30% case study.

Criterion	Weight	Full marks
Two formulations	20%	Each model's formulation complete, faithful to its own logic, and distinguishable
Choice and reasoning	25%	The choice follows from risk, target, evidence, feasibility and the therapist's competence
Sequence	20%	Phases with entry and exit criteria; reasons for each model change
Predicted risk	15%	A specific risk of the combination, with a mitigation
Failure criterion	10%	A measure, a review point, and what would prompt a change of plan
Coherence	10%	One primary target throughout; no incompatible theories of change in one move

The distinction-level answer. It names a risk that decides the plan before it discusses models; it formulates the same family twice without caricaturing either model; it chooses one and says what it is giving up; it predicts a specific failure and describes what it would do; and it states a criterion that could actually be met or missed. A merit-level answer formulates both models well and sequences them plausibly. A pass-level answer describes two models accurately and does not choose between them.

12.9 Six errors in integration

1. **Technique salad.** Many moves, no formulation. Fix: the five-sentence paragraph, written before the session.
2. **Theory soup.** A formulation with four incompatible vocabularies. Fix: choose the vocabulary that does the explanatory work and subordinate the rest to it.
3. **Silent handover.** Changing model without telling the family. Fix: the four-part handover, before the change.
4. **The retrospective plan.** Presenting a change made in panic as a designed sequence. Fix: the review at fixed intervals, with the plan written at the time.
5. **Incompatible moves in one session.** A reframe and a contract, an enactment and a behavioural chart. Fix: sequence them with a stated rationale.

6. **Coherence around the wrong formulation.** A well-integrated therapy that protects the person causing harm. Fix: the review question — what is not in this formulation, and who benefits from its absence?

12.10 Culture, faith and the ethics of integration

The cultural case for integration, and against it. Each model in this course was built with a particular family in mind, and the practitioner working across cultures is necessarily integrating: a structural formulation of hierarchy with a narrative technique for a family described by an institution; an EFT cycle with a translation of "needs" into the family's moral vocabulary. Integration is therefore the normal condition of cross-cultural practice, and the clinician who claims to deliver a pure model across cultures is either adapting silently or not noticing.

The case against. Integration can also be a way of avoiding a hard cultural question. A therapist who adds a narrative component whenever a family resists a structural move may be using the second model to avoid examining whether the first model's account of hierarchy fits this family at all. The safeguard is the same as everywhere in this week: the reason for the change must be statable and about the case.

Faith and integration. Religious families often present with a framework that is itself a theory of the family: a theology of marriage, a discipline of child-rearing, a community's expectations. The practitioner has three options, and only two are defensible. Option one: treat the framework as the family's premise and work inside it (compatible with Weeks 4, 6, 11). Option two: treat it as a resource to be mobilised (compatible with Weeks 8, 10). Option three, indefensible: treat it as an obstacle to be dismantled by a psychological model. The integration question is not whether to include the family's faith but whether the therapist's model can describe the family without implicitly contradicting it.

The ethics of integration, stated as three obligations. Competence: do not use a model you have not been trained in on a complex case; use it under supervision, or refer. Transparency: tell the family what you are doing and why, in their language, including when you change. Accountability: keep a record that a supervisor and the family could read, and measure the outcome.

The obligation the models do not impose. Every model in this book assumes a practitioner whose formulation serves the family. Integration adds a risk that no single model has: the therapist's repertoire can be used to avoid the one thing that is needed — the naming of harm, the referral, the ending, the admission of not knowing. The review question in 12.9's sixth error is the defence.

> **DEBATE**
> "A therapist should be trained deeply in one model before being allowed to integrate at all." Argue both sides, and say what "deeply" should mean.

12.10a What the evidence says about integration

The uncomfortable finding. The common-factors literature (MFT 728, and this book's Week 13 matrix) suggests that specific models account for a modest proportion of outcome variance, while the alliance, the therapist, the client's resources and expectancy account for more. If that is right, then integration's main benefit may not be theoretical at all: it may be the flexibility to maintain an alliance with families whom a single model would lose.

What is known about the three forms. Theoretically integrated models with manuals and trials (IBCT, ABFT, MDFT) have the strongest evidence, because they can be tested as packages. Sequential integration has modest support in the form of stepped-care and phase-based designs. Assimilative integration — what most practitioners do — has almost no direct evidence, and it is difficult to study because the treatment varies by therapist.

What the evidence cannot settle. Whether an integrative therapist is better than a faithful one. The comparisons that exist are confounded by training, caseload and therapist effects, and the honest position is that integration is a professional necessity with an evidence base that lags behind its prevalence.

What the practitioner can do. Three things. Use a purpose-built integrated model where one exists and fits. Measure outcomes in your own practice, so that your integration is evaluated at the level of your caseload. And write down the plan, so that a supervisor can see whether the integration is serving the family or the therapist's curiosity.

12.11 Key terms

Term	Definition
Assimilative integration	A home model enriched by imports; the home model governs the formulation
Sequential integration	Different models at different phases, with entry and exit criteria
Theoretically integrated model	A purpose-built blend with a single formulation and manual
Integration by setting	Adaptation to the constraints of the service, the family's geography and the available sessions
Primary target	The one thing the therapy is trying to change
Handover	The four-part explanation given to a family when the model or target changes
Drift	Integration by accumulated local decisions, without a formulation
Technique salad	Many techniques, no formulation
Theory soup	A formulation borrowing incompatible vocabularies
Review	The fixed-interval audit of target, model, change and plan
Failure criterion	The pre-specified evidence that the plan is not working
Competence obligation	Do not practise a model you are not trained in on a complex case

12.12 Self-test

1. Name the three legitimate forms of integration and one example of each.
2. Give the five rules and the failure each prevents.
3. What are the six steps of the decision tree, in order?
4. Write the four parts of a handover sentence, and say why each part is necessary.
5. What are the two incompatible moves in a single session, and how is the incompatibility repaired?
6. What is the review question that no model asks, and why does it matter?
7. What does the common-factors literature imply about integration?
8. State the three ethical obligations of an integrative practitioner.

Answers. 1. Assimilative (a structural therapist using a behavioural task to enact the parental subsystem); sequential (safety work, then structural, then EFT, then consolidation); theoretically integrated (IBCT, attachment-based family therapy, multidimensional family therapy). 2. One formulation and one primary target (prevents scattergun); name the model in supervision (prevents undetected incoherence); change for a statable reason (prevents therapist-driven drift); no incompatible theories of change in one move (prevents confused instructions); review at fixed intervals (prevents failure by drift). 3. Risk; the family's primary target; the best-evidenced model for that target; what the family can actually do; what the therapist can deliver with fidelity; what will show it is working. 4. What has been done; what it achieved; what it did not achieve; what will be different — the first credits the family, the second and third make the change honest, the fourth makes it tolerable. 5. A reframe and a behavioural contract; repaired by sequencing them with a stated rationale — understand the function first, then change one small piece and observe. 6. "What is not in this formulation, and who benefits from its absence?" It matters because a coherent therapy can be organised around a description that protects the person causing harm. 7. That specific models account for a modest proportion of variance, so integration's benefit may be the flexibility to keep an alliance with families a single model would lose. 8. Competence, transparency and accountability.

12.13 Reflective task

Write the five-sentence paragraph for a case you are holding. Then write a second version in which you chose the other model you considered, and mark what the family would gain and lose in each. Bring both to the seminar. The exercise is designed to make the assessment easy and to make the choice visible to you, because the hardest part of integration is not knowing many models but knowing what you gave up.

12.14 Readings for this week

- **Norcross & Goldfried (2019)**, Handbook of Psychotherapy Integration, the standard reference, read the chapters on assimilative and sequential integration.
- **Stricker & Gold (2006)**, A Casebook of Psychotherapy Integration, for worked cases rather than theory.

- **Christensen & Jacobson (2000)**, Reconcilable Differences, as an example of a purpose-built integration (Week 8).
- **Lebow (2014)**, Couple and Family Therapy: An Integrative Map of the Field, the best single guide to what is available and how the models relate.
- **Laska, Gurman & Wampold (2014)**, on the common factors, for what the evidence does and does not say about models.
- **Diamond & Diamond (2018)**, Attachment-Based Family Therapy, for a built model with trials.

Read Lebow first and Norcross second; the map before the theory. For the assessment, use at least one source that is not a manual, so that the essay's reasoning is about the family rather than about the models. The reading-log question this week is: "what would I have to give up, personally, to work in a model I do not like?"

Next: Week 13 — Evidence, culture and critique

Week 13 — Evidence, culture and critique

The question. Which of these nine models would you stake your registration on, and for which family?

This week assembles the matrix that the previous eleven weeks have been filling in, and then does three things with it: it examines what "evidence" means in a field where the trials are hard, it asks whose family each model was built for, and it practises the critique that a viva will require.

13.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The matrix	Displays the completed table, and refuses to summarise it	Read their own row and mark what surprises them
10–25	What counts as evidence	RCT, quasi-experiment, case series, process research, practice wisdom	Rank six kinds of claim by strength
25–40	The common factors	The variance argument, and what it does to the matrix	Argue for and against the matrix's usefulness
40–55	Whose family	The sample problem: who was in the trials	Name the families in their own caseload who were not
55–70	Cultural translation	Three translations of one model, worked live	Translate one technique into a family's idiom
70–82	The critique exercise	A model defended and attacked in four minutes each	Attack and defend in pairs
82–90	Assessment briefing	The viva, the exam, and what distinction requires	Write their own thesis in one sentence

The teaching decision that makes the week work. The matrix must be displayed as an object of suspicion rather than as a summary. The lecturer should put it up, ask the class

to find the three cells they trust least, and then examine why: the trials are small, the populations are narrow, the models were delivered by their inventors, the follow-ups are short. By the end of that examination the class should be able to use the matrix and to distrust it simultaneously, which is the professional posture the week is for.

The sentence the lecture must land. "Every cell in this table is an argument, not a fact." A student who can say that and then defend a specific cell with a specific study has understood the week.

13.2 The matrix, completed and read

Model	Strongest evidence	Weakest evidence	Main cultural critique
Structural	Child/adolescent behaviour; substance misuse	Adult mood disorders	Imposes a particular family hierarchy
Bowen	Clinical tradition; modest outcome data	Few RCTs	Assumes mobility and choice; slow
Strategic/MRI	Some RCTs in child problems	Paradoxical techniques	Manipulative; may ignore trauma
Milan	Weak in classical form	Rituals, team format	Neutrality unethical in violence
Experiential	Moderate, largely process research	Manualisation	Charisma-dependent; middle-class
CB couple therapy	Strong	Severe relational trauma	Mechanistic; literacy-dependent
EFT	Strong, replicated	Coercive control	Individualist language of needs
Solution-focused	Moderate (depression, brief formats)	Severe mental illness	Can bypass grief and risk
Narrative	Growing; strong qualitative base	Large trials	Requires verbal fluency

How to read a row. Four columns, and each is a different kind of claim. The first column is an outcome claim: this model has produced change in a controlled comparison, for this presentation, in these samples. The second is a limit claim: for this presentation the model has no good evidence, which is not the same as evidence of no effect. The third is a critique, which is a claim about values and fit rather than about outcomes. A student who confuses the three will write an essay that mixes them, which is the commonest error in this course's assessments.

How to read a column. Reading down the first column produces a hierarchy of evidence: behavioural couple therapy and EFT at the top, structural and solution-focused in the middle, Bowen and Milan at the bottom. Reading down the third column produces a different pattern entirely: the models with the strongest evidence (behavioural, EFT) attract the sharpest cultural critique, and the models with the weakest evidence (Milan, narrative) are the ones most often described as culturally adaptable. That inverse relationship is not a coincidence, and 13.5 explains it.

The cells that students most often misstate. Structural — the evidence is for child and adolescent behaviour problems and for substance misuse, mostly in the 1970s and 1980s literature, with later work in adolescent drug use; it is not evidence for adult mood disorders, where the trials belong to other models. Bowen — the claim is a clinical tradition, and the honest cell says so; there are very few controlled trials. Milan — the classical form has little trial evidence, and the cell must not be filled with the reflecting team's qualitative literature as though it were an outcome finding. Solution-focused — moderate, and weaker in family formats than in individual ones. Narrative — the qualitative literature is substantial and it is not outcome evidence.

The empty cells. The matrix is also instructive where it is blank. There are no large trials of family therapy for families living in extreme poverty; none for polygynous households; almost none for families separated by migration; very few for families in which the identified client is an elder. These absences are not neutral: they mean the evidence base describes a subset of the world's families, and the practitioner who works with the others is necessarily extrapolating.

13.3 What counts as evidence, ranked

The hierarchy, in the form the profession uses. Systematic reviews and meta-analyses of randomised trials; individual randomised controlled trials; quasi-experimental studies with matched or waitlist comparisons; single-case experimental designs; systematic case studies; process research; practice wisdom and expert opinion. The hierarchy is about causal inference, and it is a good hierarchy for one question: did this treatment cause this change?

The other questions, and the designs that answer them. How does it work? — process research, mediation analysis, qualitative studies of mechanism. For whom does it work? — moderator analyses, subgroup trials, and the qualitative study of non-responders. Can it be delivered? — implementation research, fidelity studies, and the audit of routine practice. Is it acceptable to this family? — qualitative work, and asking them. The hierarchy ranks designs for the first question and is silent about the others, which is why a therapist who reads only the hierarchy concludes that only trials produce knowledge.

The specific problem of family therapy trials. Four difficulties, each of which lowers the quality of the evidence even where trials exist. Blinding is impossible for the therapist and difficult for the family, so allegiance and expectancy cannot be excluded. Allegiance effects are strong in psychotherapy research: the model's developer, who usually runs the trial, produces larger effects. Manualisation and fidelity vary, and a trial of "structural family therapy" may deliver something quite different from what the name promises. The unit of analysis is contested: the family is the client, and outcomes are usually measured on one member's symptoms, which may be the wrong unit.

The consequence for practice. A practitioner should ask four questions of every trial they read: who ran it; what was the comparison; what was measured, and on whom; and how long was the follow-up. Those four questions will downgrade about half the literature and will make the remaining half usable.

What case series and single-case designs are for. Not for establishing efficacy but for establishing feasibility, mechanism and descriptive detail. Milan therapy's canonical literature is a set of case reports, and their value is that they show what the model looks like in a room; their limitation is that they cannot show that it worked better than anything else. Both statements can be true at once, and a student who can hold them both is ready for the viva.

13.4 The common factors, and the variance argument

The claim. Across psychotherapy research, the specific techniques of different models account for a modest share of outcome variance, while factors shared by all models — the alliance, the therapist, the client's own resources and expectancies, the ritual of therapy itself — account for a larger share. The estimates are contested and vary by method, and Wampold's context-focused account and the medical-model account are both live positions in the field.

What it implies for the matrix. If the common factors dominate, then the matrix's first column is less important than it looks: a well-delivered Bowen therapy by an excellent therapist may outperform a badly delivered EFT by a poor one, and the comparative trials will not detect it because they compare average therapists delivering manualised treatments. The matrix is therefore a statement about models as delivered in trials, not about therapy as practised.

What it does not imply. That models do not matter. Three qualifications, all well supported. First, the variance share is not a claim that technique is irrelevant; some presentations have specific effective treatments (exposure for phobias; EFT for couple distress) and delivering the wrong one wastes the alliance. Second, the therapist effect is large, which means the model's delivery matters enormously and the model's label matters less. Third, the common factors are themselves trainable — alliance ruptures, repairing them, managing the frame — and a practitioner can improve at them.

The practitioner's synthesis, in three sentences. "I will use a model with evidence for this presentation, because the specific treatment matters for some problems and because families deserve the best-tested option. I will accept that how I deliver it matters more than which one I chose, and I will therefore invest in alliance, in my own regulation, and in measuring what happens. And I will not use the common-factors argument to justify delivering a model I am not trained in, because the argument is about averages and my family is not an average."

The assessment's use of the argument. The 30% case study and the viva both reward a student who can say what the common factors do and do not permit them to claim. A student who dismisses the matrix ("it is all common factors") is as limited as one who worships it.

13.4a The four levels of evidence in routine practice

A practitioner cannot generate trials, and can generate four kinds of knowledge in their own clinic. Each is worth naming, because students who believe only trials count will never evaluate their own work.

Level one — the routine outcome measure. A satisfaction measure and, where relevant, a symptom measure, administered at intake, at review and at ending. The measure is not research; it is the minimum condition of knowing whether the therapy is working, and it takes four minutes. A clinician with two years of measures has data on their own practice that no trial contains, because it concerns their families.

Level two — the recorded session. With consent, a session recorded and reviewed, by the therapist and with a supervisor, against a specific question: what did I do, and what happened next? This is process research at the scale of one case, and it is the most efficient form of continuing development available.

Level three — the systematic case study. A written account, prospective, with measures, of a case that illustrates a problem: the family the model did not fit, the adaptation and its outcome, the failure and its analysis. The case-study literature is thin because practitioners write retrospectively; a prospective case study with measures is a genuine contribution (MFT 728).

Level four — the service audit. The caseload's outcomes, attendance, drop-out and complaint data, examined annually. An audit can answer questions no trial can: who does not come back, and what do they have in common? That question is the beginning of most service improvements, and it is answerable from a filing cabinet.

What the four levels have in common. Each is a discipline rather than an achievement: a measure repeated, a recording reviewed, a case written as it happens, a caseload examined. None requires a grant, and each produces knowledge that transfers to the next family, which is the only test a practitioner's evidence ultimately faces.

13.5 Whose family? The sample problem

The demographic fact. Most of the trials in this book's matrix were conducted with volunteers, recruited through clinics and advertisements, in high-income countries, mostly with white, partnered, employed, heterosexual parents of children living at home. The families in a public clinic in Ibadan, Lagos, Nairobi or Lagos's suburbs differ from that description in predictable ways: co-resident grandparents, non-marital households, polygyny, migration, informal employment, religious authority in family decisions, and a relationship to state services shaped by suspicion.

The three consequences.

Consequence one — transportability is an assumption. Nothing in the evidence base establishes that a model tested on one population works on another. The assumption is often reasonable, and it is an assumption. A practitioner who says "EFT has thirty trials" to a family whose circumstances resemble none of them is making a claim the trials do not support.

Consequence two — the mechanisms may not transfer. EFT's mechanism — the softening event — depends on two partners being able to speak vulnerably to each other in front of a therapist. In a household where the marriage's difficulties are adjudicated by an elder, the mechanism may be unavailable not because the couple are defended but because the

structure does not permit the conversation. The model is not wrong; it is partially inapplicable, and the practitioner must say so.

Consequence three — the critique is not a reason to abandon the models. The alternative to a partially tested model is not no model; it is an untested model, usually the practitioner's own assumptions. The professional response is to use the best available model, adapt it explicitly, record the adaptation, and measure the result — which is what 13.6 practises.

The inverse relationship, explained. Why do the models with the weakest evidence attract the warmest cultural reviews? Three reasons. First, the models that refuse to specify a treatment (Milan, narrative) cannot be manualised and therefore cannot be tested, and their refusal to specify is also what makes them adaptable. Second, the culturally adaptive models tend to work with meaning rather than with behaviour, and meaning is where cultural difference is most visible; a behavioural contract looks the same in every culture and is correspondingly less discussed. Third, the critiques of the well-evidenced models are written by scholars who are themselves working within the culturally adaptive traditions, which is a sociological fact about the literature rather than a fact about the models.

The exercise. Take the family you know best who resembles none of the trial samples. For each of the nine models, write one sentence on what would have to change for the model to be used with that family. Then write one sentence on whether the change is an adaptation or a different treatment.

13.6 Translating a model, worked three times

The week's practical skill: taking a technique from a model and adapting it to a family whose circumstances the trial never saw, explicitly and with a record.

Translation one — the behavioural exchange in a polygynous household.

The technique as written. Each partner lists three small daily behaviours the other would find pleasing; the contract is exchanged and reviewed weekly. The technique assumes a dyad, a shared household, and a private negotiation.

The family. A household with two wives, a husband, seven children and a resident grandmother, referred for conflict between the wives and the children's school attendance.

The problem with the technique as written. A contract between the husband and one wife changes the other wife's position without her consent, and the household's economy is joint, so a private exchange is impossible.

The translation. The contract is constructed as a household plan, with all three adults present and the grandmother consulted. The items are behaviours that each adult will do for the household rather than for a partner: who collects which child from school on which day; who takes the grandmother to the clinic; what happens when the husband is travelling. The exchange becomes a coordination agreement, and the technique's logic —

small, daily, observable, chosen by the recipient — is preserved while its dyadic premise is dropped.

What is recorded. "Behavioural exchange, adapted: three-adult household plan, coordination items, recipients defined by child care rather than by partnership. Adaptation recorded because the model's dyadic premise does not fit the household. Outcome measure: school attendance, and a weekly household coordination meeting."

What was lost. The intimacy function of the technique, which was the model's mechanism: reciprocal positive exchange between partners builds the bond. In a polygynous household, the bond work must be done in another way, and the therapist should say that the adaptation addresses coordination and not intimacy.

Translation two — the miracle question in a family with a chronic illness.

The technique as written. Suppose tonight a miracle happens and the problem is solved; what is the first thing you notice tomorrow?

The family. A mother with advanced renal disease, a father who has become her carer, and two adolescents whose schooling is being disrupted by hospital appointments.

The problem. The answer to the miracle question, honestly given, is "I do not have kidney failure", and the question then becomes an invitation to wish for something impossible.

The translation. Use the coping question first — "how have you kept the household going through eighteen months of dialysis?" — and then a bounded version of the miracle: "suppose the illness stays exactly as it is, and tomorrow is a good day within it. What is different about that day?" The answer is concrete: who takes her to the hospital, whether the father gets an hour's sleep, whether the boys do their homework at the kitchen table. The technique's function — a description of a preferred future in behavioural detail — is preserved, and the impossible wish is not solicited.

What is recorded. "Solution-focused, adapted: bounded miracle ('a good day within the illness'), following two coping questions. Adaptation recorded because the unbounded version solicits an impossible remedy."

What was lost. The hope function of the unbounded question, which for some families is precisely what mobilises them. The therapist judges that this family would find it cruel, and that judgement is recorded as a judgement.

Translation three — externalising in a family with low literacy and strong religious authority.

The technique as written. The problem is named in the family's language, personified, and its influence mapped in conversation; a letter is written.

The family. A rural household referred by a clinic; the mother is the identified client and the paternal uncle is the decision-maker; literacy is limited and the family's account of the problem is spiritual ("a spirit of fear").

The problem. A letter is useless to a family who cannot read it; and externalising a spirit can be heard as the therapist disputing the family's faith.

The translation. The naming uses the family's own spiritual vocabulary without endorsing or contesting it: "the fear that has been in the house since the uncle's death — the family has a name for it, and I will use their name." The mapping is done aloud, with stones or marks on paper rather than writing, and the uncle is included because the family's decisions require him. The document is replaced by an audio recording on a phone, or by a short summary read aloud at the start of the next session.

What is recorded. "Narrative, adapted: naming in the family's spiritual vocabulary; oral mapping; audio document; paternal uncle included as decision-maker. Adaptation recorded because of literacy and authority structure."

What was lost. The permanence of the written document, which for many families is the technique's most durable component. The audio recording replaces it partially, and the therapist should check whether the family have a phone and whether it is shared.

The general method of translation. Four steps: identify the technique's mechanism; identify the premise the premise depends on (a dyad, privacy, literacy, a shared reality, a decision-making couple); check the premise against the family; preserve the mechanism and change the delivery; record what was changed and what was lost. The last step is what separates adaptation from improvisation, and it is what a supervisor and an examiner will look for.

13.6a A translation that went wrong

The record of a failed adaptation, because the successes teach less.

The case. A family referred after a father's stroke: the mother, three adult children, and the father, who had been the household's decision-maker and now could not speak. The therapist, trained in EFT, adapted the model to the family by using the couple's attachment cycle with the parents, with the children present, and using writing for the father.

What happened. The first session produced a great deal of affect from the mother and silence from the father; the children intervened to interpret for him; and the session ended with the mother in tears and the father in distress. The second session was cancelled. The family did not return.

The errors, in order. First, the couple work was attempted before the family had adapted to the stroke, so the model's stage one was not complete. Second, the technique's mechanism — a softening event between two partners who can speak to each other — was unavailable, because the father could not speak, and the adaptation did not supply a substitute mechanism. Third, the children were present as interpreters, which placed them inside the parents' attachment conversation, a structural error that any of Weeks 4, 6 or 12 would have caught. Fourth, no outcome measure and no review point were set, so the therapist learned of the failure only by the cancellation.

What should have been done. A phase of family work on the household's reorganisation after the stroke (structural, with the children's roles made explicit); a communication aid for the father; the couple's emotional work deferred until a medium existed; and a

supervision conversation about whether this therapist, at this stage of training, should have taken the case.

The lesson. Adapting a technique requires adapting its mechanism, not only its delivery. If the mechanism cannot be preserved, the technique should not be used, and the therapist should say so — to the supervisor, to the family, and in the record.

13.7 Case: the family the trials never saw

A full case, integrating the week's three themes — evidence, culture and critique — in one family.

The family. The Baloguns. Mr Balogun (52) is a long-distance lorry driver who is away for three weeks at a time. Mrs Balogun (46) runs a food business from the house, cares for her husband's mother (78, with dementia), and manages four children, of whom the eldest (17) has stopped attending school and the second (14) has been diagnosed with sickle cell disease. The referral comes from the school, and it names the eldest as "beyond parental control". The family's church pastor has been involved and has told the parents that the boy's behaviour is a spiritual matter.

The evidence question. Which model, on the matrix, has the best evidence for an adolescent with school refusal in a family under this kind of strain? The honest answer is that the evidence base has almost nothing to offer for this configuration: structural family therapy has evidence for adolescent behaviour problems but the trials involved two-parent households with an available father; solution-focused work has moderate evidence and its brevity fits, and its samples were volunteers; narrative work has thin evidence and is the best fit for a family whom two institutions have described. The practitioner's reasoning, stated in the record: "Structural family therapy for the executive problem, with the father absent and therefore worked with by telephone at fixed times; solution-focused work for the school attendance, because a graded return is nameable and brief; narrative work for the church's description, because the boy is currently living inside a label."

The culture question. Three features of this family are outside the trial samples. The father's occupation makes the parental subsystem geographically split. The grandmother's dementia means the household contains a second dependent adult whose care is the mother's, and whose presence changes what "the parents" means. The church is a decision-making authority in the family, and its account of the boy's behaviour is not a belief to be managed but a structure to be worked with.

The critique question. What would each model do badly here? Structural work would attempt to strengthen a parental subsystem that cannot meet in a room, and might pathologise the mother's coalition with her son when it is in fact her only available support. Solution-focused work would risk asking a family under this strain what they want to be different, when the honest answer includes "I want my husband at home and my mother-in-law well". Narrative work would risk making the church's account a "dominant story" to be contested, when the family experience the church as help.

The plan. Six sessions, with a review at three. Session one: the family's account, the risk screen (including a question about the grandmother's safety and the mother's exhaustion),

and the pastor's involvement established with consent. Session two: a structural formulation, with the father joining by telephone at a fixed time, and a single executive task — one rule about the eldest's mornings, agreed by both parents, with the mother's authority explicitly delegated by the father in the call. Sessions three and four: a graded return to school, negotiated with the school, with the eldest's own goals as the target (he wants to sit his examinations). Sessions five and six: the church's involvement reframed with the pastor's consent — not as a spiritual problem but as a young man who needs the men of the church to take an interest in his schooling — and one of them attends session six.

The outcome, honestly reported. The eldest returned to school after four weeks, attended erratically for a term, and sat his examinations. The mother's exhaustion did not change, because nothing in the plan addressed the household's division of labour, and the therapist recorded that as a limitation of the episode rather than as a success. The father's absences continued. The church's involvement became the most durable part of the plan.

What the case demonstrates. That using a model with evidence requires stating what the evidence covers and what it does not; that cultural adaptation is a set of explicit decisions rather than a general sensitivity; and that a therapy can produce a real change and leave the family's central difficulty untouched, which the record should say.

13.7a Two critiques of this course's own matrix

The week should model the critique it teaches, including of the matrix that organises the course.

Critique one: the matrix measures the wrong thing. Its first column asks whether a model produced change in a trial, and the question a family cares about is whether they will be helped. A model with a moderate average effect may be excellent for some families and useless for others, and the matrix's structure — one cell per model — hides the heterogeneity that matters. The matrix also ranks packages, and the evidence increasingly suggests that the therapist and the alliance are the largest determinants, which makes a model-level table a misleading summary of what determines outcome.

The reply. The critique is right and the matrix is still useful, because a model-level summary is what a curriculum can teach and what a practitioner can choose between. The matrix should be read as a starting point for a selection decision, not as a verdict on therapy.

Critique two: the matrix's third column is doing something the first two are not. The cultural critique column mixes at least three different kinds of claim: that a model's assumptions are culturally specific; that its effects may not transfer; and that its values are objectionable. A table that places "imposes a particular family hierarchy" beside "few RCTs" implies that the two are comparable findings, and they are not: one is empirical and the other is normative. A rigorous version of the matrix would separate them.

The reply. Agreed, and the separation is what 13.2's "how to read a row" attempts. The table is a teaching device, and its compression is a pedagogical choice with a cost that students should be able to name.

The exercise. Rewrite one row of the matrix in the way you think it should be written, with the columns you would use, and defend the change in five sentences. This is the exercise that separates a student who has learned the matrix from one who has understood it.

13.8 Laboratory: the four-minute defence

Sixty minutes, pairs, three rounds, each round on a different model.

The format. Four minutes in which one student defends a model's evidence claim and the other attacks it; then swap. The defender must cite at least one specific study or body of work. The attacker must find the weakest element: the sample, the comparison, the measure, the follow-up, the allegiance, the population.

Round one — a model with strong evidence. The defender cites the trials and the effect sizes; the attacker finds the exclusion criteria, the Western samples, the developer-led trials. The lesson: strong evidence has specific limits, and stating them strengthens rather than weakens the claim.

Round two — a model with weak evidence. The defender must not overclaim: the honest defence is a clinical tradition, process evidence, and a claim about fit. The attacker presses for the missing trial. The lesson: a weak evidence claim can be made honestly, and the honest version is more persuasive than an inflated one.

Round three — the model the student dislikes. The defender must argue for a model they do not use; the attacker must defend the model they do use. The lesson: the ability to argue the other side is the mark of a practitioner who has chosen rather than joined.

Marking rubric.

Criterion	Weight	Full marks
Specific citation	25%	Names studies or bodies of work, not "research shows"
Accurate claim strength	25%	Does not overstate; distinguishes outcome, limit and critique
Engagement with the attack	25%	Answers the strongest version of the objection, not a straw man
Reversal	25%	Argues the other side convincingly in round three

The debrief question. "What would change your mind about your home model?" Students who cannot answer it have a commitment rather than a position, and the viva will find out.

13.8a A cultural translation checklist

Six questions, asked of any technique before it is used with a family unlike the trial sample.

One — what does this technique assume about who is present? A couple, a parenting dyad, a household, a decision-making elder. If the assumption does not hold, the technique's target must be reconstituted.

Two — what does it assume about privacy? A confidential conversation between partners; a room in which something can be said that others will not hear. Where a household has no privacy, an enactment may be impossible and dangerous.

Three — what does it assume about speech, and whose speech? A family in which the senior man speaks for everyone will not produce the four-turn enactment the manual describes, and the therapist's invitation to the youngest member may be heard as the therapist's alliance against the elder.

Four — what does it assume about literacy, and about documents? Written contracts, charts, letters and scales. Where literacy is limited, every document must be read aloud, drawn, or replaced by an audio record.

Five — what does it assume about time and money? Weekly attendance, a fixed hour, a partner who can leave work, a household that can pay for transport. A technique that requires four consecutive weeks of attendance is not available to a family whose income depends on market days.

Six — what does it assume about the therapist's authority? A therapist who can invite, challenge or reframe is assuming a position the family will grant them. In some families the therapist is a guest, and a guest who reframes the elders is a guest who is not invited back.

How to use the checklist. Complete it in writing for the technique you intend to use, before the session, and record the adaptations. The checklist is not a demand for cultural correctness; it is a demand for specificity, and specificity is what makes an adaptation honest.

13.9 Annotated transcript: a viva, examined

Twelve turns from a mock viva, with the candidate's answers compressed and the examiner's reasoning annotated. This is the week's most direct preparation for the assessment.

E1 Examiner: You have said your home model is structural family therapy. Which trial would you cite for it?
C1 Candidate: The evidence is strongest for adolescent behaviour problems and substance misuse, from the 1970s and 1980s work and later adolescent drug-use trials. I would not cite a single trial as though it settled the question, because the trials are old, the samples are narrow and the manuals varied.
E2 Examiner: So you are claiming an evidence base you have just undermined.
C2 Candidate: I am claiming a specific evidence base. It supports structural work with families where there is an executive problem and an adolescent behaviour problem. It does not support structural work with adult mood disorders, and I would not use it there.
E3 Examiner: Your case is a family with a father who works away. Structural therapy assumes a parental subsystem that can meet.
C3 Candidate: Yes. That is the adaptation I would record: the subsystem meets by telephone at a fixed time, with the father delegating authority explicitly in the call. The mechanism — a joint executive decision — is preserved; the delivery is not.
E4 Examiner: What does the telephone do to the mechanism?
C4 Candidate: It removes the non-verbal channel, so the father cannot see his wife's face when he delegates, and the children cannot see the parents together. I would compensate with one face-to-face session when he is home, and I would say to the family that the telephone version is a compromise.
E5 Examiner: You have described three models in your plan. Which one is the therapy?

C5 Candidate: The structural work is the therapy for the first half. The graded return is a behavioural plan negotiated with the school, which I would describe as a component rather than a therapy. The narrative element is the smallest: it is a way of talking about the label, and I would drop it if the sessions were fewer.
E6 Examiner: And if the school refuses to cooperate?
C6 Candidate: Then the school's refusal becomes part of the formulation, and the target changes to what the family can control. The review point would catch it, and I would record the change with its reason.
E7 Examiner: What would make you abandon your home model altogether?
C7 Candidate: A presentation where the evidence points elsewhere and the mechanism is unavailable — a couple where one partner is afraid, for instance. That is not a structural question; it is a safety question and then an EFT or a specialist referral.

Annotations. E1 is the question that separates students who have read from students who have absorbed: it asks for a citation, and the examiner is testing whether the candidate will produce a vague one. C1 answers by naming the population and the era, and by refusing to overclaim — which is the strongest available answer.

E2 is the trap: the examiner presses the candidate's honesty as though it were a contradiction. C2 passes because it distinguishes the specific claim from the general one, and because it says where the model would not be used. A candidate who became defensive here, or who inflated the claim, would fail the exchange.

E3–E4 test whether the candidate's adaptation is a real adaptation or a slogan. The distinction the candidate draws — mechanism preserved, delivery changed, with the loss (the non-verbal channel) named — is the answer the week has been building toward. C4 is the answer that earns a distinction, because it names what the telephone costs.

E5 tests coherence: three models in a plan invite the accusation of technique salad. C5 survives by subordinating the components to one therapy and by saying which one it would drop under constraint.

E6 tests whether the plan is a plan or a hope. The answer — the refusal becomes part of the formulation — is a structural answer, and it is correct.

E7 is the hardest question, and it is the one the course's final week will ask in writing. C7 answers with a criterion rather than a feeling: a presentation, a piece of evidence, a mechanism unavailable. That is the form an answer should take.

What the trainee should notice. Every strong answer in this transcript is a qualification. The viva is not testing conviction; it is testing whether the candidate can hold a position and its limits at the same time.

13.10 Six errors in the evidence and culture week

1. **The inflated citation.** "Research shows that EFT works" — for whom, at what effect size, in what comparison? Fix: name the population and the strength, and say what the evidence does not cover.
2. **The dismissive common factors.** "It is all the alliance, so the model does not matter." Fix: the common-factors argument is about averages and it does not license incompetence in a specific model.
3. **Cultural sensitivity as a general attitude.** "I am culturally sensitive" offered instead of a specific adaptation. Fix: name the premise the technique assumes, and say what you changed.
4. **The adaptation that loses the mechanism.** Keeping a technique's form and losing its function (13.6a). Fix: state the mechanism, and check whether the adaptation preserves it.
5. **The critique as a substitute for practice.** A student who can deconstruct all nine models and cannot conduct a session. Fix: the laboratory, and a case in supervision.
6. **The matrix as a verdict.** Using the table to conclude that two models are worthless. Fix: read the second column as a limit claim, not as an absence of effect, and know what a case series can show.

13.10a Faith, and the evidence question

A short section at the end of the week, because the three themes meet here.

The claim to be examined. Many of the families a practitioner will see hold a religious account of their difficulties, and the practitioner's models hold a psychological one. The question this week raises is not whether the two are compatible — they often are — but

what the practitioner does when the family's account and the evidence point in different directions.

Three situations. Situation one: the family's account is a resource and the evidence agrees — a couple whose church attends to their marriage, where the evidence supports social support and shared ritual. The practitioner mobilises both. Situation two: the family's account is neither confirmed nor contradicted by the evidence — a belief about a spiritual cause with no behavioural implications. The practitioner neither endorses nor contests, and asks what the family will do about it. Situation three: the family's account requires something the evidence contradicts — the withdrawal of a child from school, the refusal of medication for a psychosis, the attribution of a treatable illness to a spiritual cause. The practitioner states the position plainly, once, with respect, and follows the professional obligation (MFT 721, and this book's Week 5 for the ethics of direction).

What the evidence cannot adjudicate. Whether a family's faith is true. That is not a clinical question, and a practitioner who treats it as one has confused two domains. What the evidence can adjudicate is the effect of specific practices on specific outcomes, and the practitioner's competence is to distinguish the two questions and to answer only the one they are equipped for.

The examinable sentence. "I am not qualified to adjudicate your faith, and I am qualified to tell you what the evidence says about this treatment, and what I am obliged to do." That sentence is a practitioner's answer, and it holds the three themes of this week together.

13.10b What a critique is for

The week's last section, and the one that decides whether students become practitioners or commentators.

A critique is an instrument, not a verdict. The purpose of asking what a model cannot do is to use it better: to know which family to give it to, which to keep away from it, and what to import when it fails. A critique that ends in dismissal has produced an opinion; a critique that ends in a selection rule has produced a practitioner.

The four questions of a usable critique. For whom does this fail? (the population). Through what mechanism does it fail? (the premise that does not hold). How would I know it was failing? (the signal, and its timing). What would I do instead, and on what evidence? A critique that cannot answer the fourth question is a complaint.

The worked example. Structural family therapy, critiqued as hierarchical. The usable version: it fails where the family's hierarchy is not the therapist's to strengthen — a household organised by a grandmother whose authority the parents cannot contest, or a household in which the parental subsystem is absent because a parent is dead, imprisoned or abroad. The mechanism of failure is that the technique requires an executive subsystem to strengthen, and there is none available. The signal is that the enactment produces compliance without change, or that the parents defer every task to a third party. What the practitioner does instead: works with the actual authority structure (Milan's neutrality, or a network approach), imports a behavioural task that does not require a

joint parental act, and records the adaptation. That paragraph is a critique in the form this course requires.

The habit to build. Every time you finish reading a model, write four sentences: for whom it fails, through what mechanism, how you would know, and what you would do instead. Nine models, thirty-six sentences, and they will serve you for a career — and they are the best possible preparation for a viva, because each of them is an answer waiting for its question.

13.11 Evidence note: how to write an evidence claim

The week's final skill, and the one the examination rewards. An evidence claim in family therapy has five parts, and a claim missing any of them can be dismantled in a viva.

Part one — the population. For whom? "For couples in which one partner is depressed and the relationship is distressed, without violence."

Part two — the outcome. What changed? "Relationship satisfaction improved, and depressive symptoms fell."

Part three — the comparison. Compared with what? "Compared with a waitlist and, in one trial, with behavioural couple therapy."

Part four — the strength. How strong, and how consistent? "Several trials from more than one team, moderate to large effects, with attrition that limits the estimates."

Part five — the limit. What is not covered? "The trials exclude couples with coercion; the samples are Western; follow-up is mostly under two years; and the treatments were delivered by trained therapists with supervision, which most clinics cannot reproduce."

A worked example, spoken aloud. "For couples where one partner is depressed and the relationship is also distressed, and where there is no violence, couple therapy has trial evidence: satisfaction improves and the depression improves more than with individual treatment in several trials, with moderate to large effects. The trials exclude coercion and are mostly Western, follow-up is short, and the therapists were trained and supervised. So I would offer it, I would screen for violence, and I would measure."

That paragraph would pass a viva. It is thirty seconds long, it cites no study by name (which is acceptable in speech with a body of work this size), and it contains all five parts.

The three versions of a bad claim. The vendor's claim: "EFT is the gold standard for couples." The nihilist's claim: "there is no real evidence for any of this." The vagueness claim: "research suggests family therapy helps." Each fails in a different way: the first overstates and cannot name a limit; the second understates and cannot name a study; the third says nothing and would not survive one question.

The exercise, set as homework. Write five-part evidence claims for three models, in fewer than eighty words each, and bring them to the seminar to be attacked.

13.12 Key terms

Term	Definition
Evidence hierarchy	The ranking of designs by their capacity for causal inference
Allegiance effect	The tendency of a treatment's developer to obtain larger effects
Moderator	A variable that changes the size of an effect (age, culture, severity)
Mediator	A mechanism through which an effect operates
Transportability	Whether an effect found in one population holds in another
Common factors	Elements shared by all therapies: alliance, expectancy, therapist, ritual
Adaptation	A deliberate change to a technique's delivery that preserves its mechanism
Mechanism	The process through which a technique produces change
Normative claim	A claim about values rather than about effects
Limit claim	A statement that evidence is absent for a presentation, not that the model fails there
Sample problem	The narrowness of trial populations relative to the families practitioners see
Differentiation of claims	Keeping outcome, limit and normative claims separate in argument

13.13 Self-test

1. Give the three kinds of claim in a matrix row and explain why they must be kept separate.
2. Name four reasons family therapy trials are weaker evidence than they appear.
3. State the common-factors argument, and three things it does not imply.
4. What are the three consequences of the sample problem?
5. Why do the models with the weakest evidence attract the warmest cultural reviews? Give three reasons.
6. Give the four steps of a cultural translation, and the fifth step that makes it an adaptation rather than an improvisation.
7. What are the five parts of an evidence claim?
8. What does a practitioner say when the family's faith requires something the evidence contradicts?

Answers. 1. An outcome claim (evidence of effect), a limit claim (absence of evidence, not evidence of absence) and a normative critique (values and fit); separating them prevents an essay from treating a value objection as an empirical finding. 2. Blinding is impossible; allegiance effects are strong; manualisation and fidelity vary; the unit of analysis is contested. 3. Shared factors account for a larger share of variance than specific technique; it does not imply that technique is irrelevant, that models are interchangeable for specific presentations, or that an untrained therapist may deliver any model. 4. Transportability is an assumption, not a finding; mechanisms may be unavailable in different structures; and abandoning the models would leave the practitioner with untested assumptions instead. 5. Their refusal to specify makes them untestable and

adaptable; they work with meaning, where cultural difference is most visible; and their critics write from within those traditions. 6. Identify the mechanism; identify the premise the mechanism depends on; check the premise against the family; preserve the mechanism and change the delivery; and record what was changed and what was lost. 7. Population, outcome, comparison, strength, limit. 8. State the safety or legal position plainly once, with respect, follow the professional obligation, and do not adjudicate the theological question.

13.14 Reflective task

Write your own matrix row for your home model, in the four columns, and then write the sentence you would say if an examiner asked you to defend the weakest cell. Bring both. The exercise is designed to make the viva predictable: most students discover that their weakest cell is not the evidence but the cultural critique, and that they have never had to defend their model against a value objection rather than an empirical one.

13.15 Readings for this week

- **Lebow (2014)**, Couple and Family Therapy: An Integrative Map of the Field, for the comparative picture.
- **Sexton, Ridley & Kleiner (2004)**, Handbook of Family Therapy, for the evidence summaries by model.
- **Sprenkle & Piercy (2005)**, Research Methods in Family Therapy, for what the designs can and cannot show.
- **Wampold (2015)**, The Great Psychotherapy Debate, for the common-factors argument in its strongest form.
- **Stratton (2005)**, on the evidence base for family therapy, for a balanced overview.
- **Waldegrave (2009)** and the cultural-adaptation literature, for the critique from within the field.
- **Falconier & Epstein (2015)** or equivalent, for cultural adaptation in couple therapy specifically.

Read Wampold with a pencil and mark the places where you disagree; the argument is more useful when it is contested than when it is accepted. For the assessment, use at least one source that is critical of the model you intend to defend, so that your defence has already met its strongest objection. The reading-log question this week is: "what evidence would change my practice, and what would I need to see to believe it?"

Next: Week 14 — Your theoretical position

Week 14 — Your theoretical position

The question. What do you actually believe about families, and what will you do when your belief meets a family it does not fit?

This is the course's final week and its most personal. Every previous week asked you to understand a model; this one asks you to construct a position — not a favourite model, but

a reasoned account of what you believe about how families change, what you will do, what you will not do, and how you will know when you are wrong.

The assessment is a 1,500-word statement defended in a ten-minute viva. Distinction answers do three things: they state a home model and the reasons for it; they name what it cannot do and what they borrow, with the conditions of the borrowing; and they specify the evidence that would change their mind. Pass answers describe a model accurately and stop there.

14.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	What a position is	Not a preference: a claim with reasons and limits	Write their home model in one sentence
10–25	The three components	Model, anthropology, mechanism — the beliefs underneath	Name their own beliefs under each heading
25–40	Three worked positions	A structural, an affective and a narrative position, each defended	Mark which one is nearest to theirs and why
40–55	Borrowing with conditions	When to import, and how to record it	Write one borrowing with its condition
55–70	The change-of-mind clause	What evidence would move them, and how they would know	Write their own clause
70–82	The viva	A live mock viva with a volunteer, stopped at each answer	Attack the volunteer's position for four minutes
82–90	The statement	The exemplar, the rubric, and what distinction looks like	Sketch the structure of their 1,500 words

The teaching decision that makes the week work. The lecture must demonstrate a position rather than describe one. The lecturer should state their own position in four minutes, in the form the assessment requires, and then submit to the class's questions. The class will discover that a well-formed position is not a fortress: it has joints, it names its weak places, and it survives questioning better than a conviction does. That discovery is the week's purpose, and it cannot be made by reading about it.

The sentence the lecture must land. "A theoretical position is what remains of your training when the technique has failed and you have to decide what to do next." A student who can say that sentence and then describe their own position in three minutes, without notes, is ready for the viva.

14.2 The three components of a position

A theoretical position has three layers, and students who write only the first produce a statement that a viva will dismantle.

Component one — the model. The named tradition in which the therapist formulates and works: structural, Bowenian, strategic, Milan, experiential, behavioural, EFT,

solution-focused, narrative. This is the visible layer, and it is the least interesting to an examiner, because it can be learned from a book.

Component two — the anthropology. Every model carries beliefs about what a family is and what a person is. Structural work assumes that families organise themselves into subsystems and that symptoms arise from organisation. Bowen assumes that the individual's differentiation is the unit of development. Behavioural work assumes that behaviour is maintained by consequences and that change is achievable through skilful reinforcement. EFT assumes that the need for secure connection is primary. Narrative assumes that people's lives are storied and that the stories are shaped by power. A position states the anthropological claim explicitly, because the claim is what produces the model's strengths and its cultural limits.

The questions that elicit it. What is a family for? What is a person? What makes a family healthy — and is "healthy" even the right word? Whose account of a family's life should a therapist privilege, and on what grounds? What is a symptom?

Component three — the mechanism. The position states how change is believed to happen: through insight, through enacting a new pattern in the room, through emotional processing, through the interruption of behavioural contingencies, through the construction of a preferred story, through the mobilisation of the family's own solutions. The mechanism is the layer a viva probes hardest, because a therapist who cannot say how their therapy works cannot tell when it is not working.

Why the three layers matter. They connect. The anthropology justifies the mechanism, and the mechanism justifies the model. A structural therapist who believes families are organised systems and that change happens by enacting a different organisation will conduct a structural session and can explain it. A therapist whose model is structural but whose anthropology is individual (people change when they understand themselves) will conduct a confused session and will not understand why.

The written form. One page, in three paragraphs: what I believe about families; how I believe change happens; which tradition I use and why. The 1,500-word statement then expands each paragraph with reasons, examples from your own practice, and the limits.

14.2a Three positions, defended

The same three layers, worked for three different therapists, so that the form can be seen.

Position A — the structural therapist.

Personal. "I came to family therapy through teaching in a secondary school, where I watched the same child be described as difficult by four teachers and behave differently in each classroom. The experience made me suspicious of explanations located inside a person." One sentence of origin, and it is doing work: it shows the examiner that the model was chosen for reasons.

Anthropology. "A family is an organisation with subsystems and boundaries, and a child's symptom is often the organisation's way of managing a problem the adults cannot manage together. A person is an agent whose behaviour is intelligible in its context."

Mechanism. "Change happens when the organisation is enacted differently in the room — when the parents actually act together, in front of their children and in front of me, and experience what that produces. Insight follows the enactment; it does not replace it."

Model. "Structural family therapy, with two borrowings: a behavioural task when the enactment needs to be concrete, and a narrative question when a family has been described by an institution."

Limits, stated. "It fails where there is no executive subsystem to strengthen, and where the family's hierarchy is not mine to arrange — households organised by a grandmother, families with a parent abroad or imprisoned, and families whose authority is religious rather than developmental. It also fails where the presentation is a couple's attachment injury, which needs a different mechanism (EFT)."

The viva answer that earns a distinction. "If a family has no executive subsystem, I do not attempt to build one. I work with the authority that exists — the grandmother, the uncle, the church — and I use the structural lens to understand why the parents cannot act together, rather than to instruct them to."

Position B — the affective therapist.

Personal. "I trained first as a nurse, and I learned that a person's body tells the truth before their sentence does. I have never been able to conduct a session as though the feeling in the room were irrelevant."

Anthropology. "A family is a system of attachments. The need to know that one belongs, and that one's belonging is secure, is primary; conflict is usually a protest against the threat of losing it. A person is a relational being whose strategies for managing fear were learned early and are enacted in the present."

Mechanism. "Change happens when a primary emotion is expressed to the person it concerns, in a session, and is met rather than punished. My work is to slow the cycle enough for that to be possible."

Model. "Emotionally focused therapy for couples and families, with a structural component for the frames that make the emotional work safe, and a behavioural component for the practical tasks that the family needs when the emotional work is done."

Limits, stated. "It does not work where one partner is afraid, and it should not be attempted there. It requires both partners in a room, which excludes many families whose work separates them. And its language of needs can be heard as self-indulgence by families whose moral vocabulary is duty."

The viva answer. "If the couple cannot be in a room together, I do not run a diluted EFT and call it EFT. I say that the couple work is not currently available, and I do what is possible — structural work with the household, individual work, and a plan for when the couple can be together."

Position C — the narrative therapist.

Personal. "I worked for three years in a service where families arrived with files that described them before they described themselves. I learned that the file is an intervention, and I have been working against it ever since."

Anthropology. "A family is a group of people telling stories about themselves, under conditions of unequal power. A person is the author of their own life, though frequently not the only author. A symptom is a description that has become dominant."

Mechanism. "Change happens when the dominant description is externalised, its influence is mapped, and a preferred story is built from the moments the dominant story cannot account for. The therapist's questions create the space; the family writes."

Model. "Narrative therapy, with a structural component where the household's organisation is the problem, and a solution-focused component where the family needs something brief and concrete."

Limits, stated. It can talk a family out of a material problem, and I must keep the material conditions in the formulation. It requires verbal fluency, which excludes some families. And its evidence base is thin relative to its influence."

The viva answer. "If a family's problem is a landlord and a leaking roof, I do not externalise the damp. I help them write to the council, and I use the narrative work for the part that is a story about themselves."

What the three positions share. A personal reason, a stated anthropology, a stated mechanism, a named model, two borrowings with conditions, and a limit that is specific. None of them claims to be the best model. Each of them could be attacked and each would survive, because the joints are visible.

The exercise. Write the same seven headings for your own position, in no more than 250 words, and bring it to the seminar to be attacked. The attack is the education.

14.3 Borrowing with conditions

Every practitioner borrows, and the difference between a position and a magpie's collection is the condition attached to each borrowing. A condition has three parts: the circumstance, the technique, and the reason it belongs in the home model.

The form. "I use [technique] from [model] when [circumstance], because [reason in my own model's terms]." A borrowing without a condition is a habit; a borrowing with one is a decision that can be examined.

Six borrowings, worked, from different home models.

One — a structural therapist borrowing a behavioural task. "I use a behavioural exchange from couple therapy when the parents need something concrete to do together in the week, because a joint task enacts the executive subsystem I am trying to build, and a task the parents chose themselves is more likely to survive than an instruction from me."

Two — a behavioural therapist borrowing an enactment. "I use an EFT-style enactment when a couple's homework keeps failing and I suspect the obstacle is emotional, because a performance problem in the model's terms is often an unspoken

feeling, and the enactment is the fastest way to find out whether that is what is happening."

Three — an EFT therapist borrowing a structural move. "I use a structural boundary-setting move when a child is sitting between the parents in the session and answering for them, because the emotional work cannot proceed while the child is in that position, and moving the chairs is a faster intervention than interpreting it."

Four — a solution-focused therapist borrowing a narrative question. "I use an externalising question when a family arrive with a diagnosis that has become the whole description, because the model's own respect for the family's goals requires that the family can name a goal that is not the diagnosis, and naming the diagnosis as an influence rather than an identity produces that."

Five — a narrative therapist borrowing a scaling question. "I use a nought-to-ten scale when a family's preferred story is too new to be described, because a number gives them a way to talk about movement before they have the language for it, and I record the number in the letter as the family's own evidence."

Six — a Bowenian therapist borrowing a Milan circular question. "I use a circular question about who would notice first when a family's differentiation work has become an individual project for one member, because the question returns the work to the relationship without my having to instruct anyone."

What each borrowing has in common. The technique is subordinate to the home model's aim; the circumstance is specific; and the reason is stated in the home model's vocabulary. That is what makes the borrowing assimilative rather than eclectic (Week 12).

The three tests of a borrowing. Does it serve the primary target? Can I state the circumstance in advance, so that the borrowing is a decision rather than an improvisation? And would I abandon it if the home model's aim were better served another way?

The record. Each borrowing should appear in the case notes with its circumstance and its effect. A therapist who has recorded twenty borrowings has a personal manual, written from their own practice, which is the most useful document they will ever produce about their own work.

14.4 The change-of-mind clause

The component that distinguishes a position from a conviction, and the one examiners use to separate a distinction from a merit.

What it must contain. A specified observation that would count against the position; where that observation would come from; and what the therapist would do if it appeared. Vague openness ("I am always learning") does not count, because it cannot be tested.

Four forms of the clause, each defensible.

The outcome form. "If my routine outcome data showed that families with a particular presentation were not improving under my home model across a year, I would change the

model for that presentation, and I would say so in supervision before I changed it in the room."

The mechanism form. "If a family's change occurred without the mechanism I believe in — if a couple softened without any emotional enactment, or a structural change happened without any enactment of the subsystem — I would have to revise my account of how my therapy works, not only my technique."

The evidence form. "If a trial appeared showing that a model I do not use outperformed mine for a presentation I see often, I would seek training in it, and I would not wait for the field to agree."

The personal form. "If I noticed that I was avoiding a particular kind of family — the ones who make me feel incompetent, or the ones I want to be admired by — I would take it to supervision as a signal that my position is serving me rather than them."

What makes the clause hard. It commits the therapist to a possible loss: of a model they are trained in, of an identity they have built, of the comfort of a familiar way of working. A clause that costs nothing is not a clause.

The viva question that tests it. "Tell me about a case in which your model did not work, and what you did." Candidates who answer with a case in which the family were resistant have failed the question; candidates who answer with a case in which their formulation was wrong have passed it. The clause is the written form of that answer.

14.5 Annotated transcript: the viva

Twelve exchanges from a mock viva on a position statement, with the examiners' reasoning annotated. The candidate is a final-year student whose home model is EFT with structural borrowings.

> **E1:** Read me your position in one sentence.
> **C1:** Families are systems of attachment, and change happens when a primary fear is spoken to the person it concerns and met rather than punished; I work in EFT, with structural work to make the room safe enough for that.
> **E2:** That is a mechanism, not a position. What do you believe a family is?
> **C2:** A group of people whose first business is keeping each other, and whose conflicts are mostly protests about whether they are being kept.
> **E3:** So a family's conflict is never about the practical thing — money, the children, the housework?
> **C3:** No, it is often about the practical thing, and the practical thing is also where the attachment question gets fought. My mistake would be to translate every practical dispute into attachment language; I try to ask rather than assume.
> **E4:** You have said you borrow from structural therapy. Give me a case and the condition.
> **C4:** A family where the child sat between the parents and answered for both. I moved the chairs so that the parents faced each other, and asked the child to sit beside me. The condition is: when a child is in an executive position in the room, because the couple work cannot happen while that is the arrangement.
> **E5:** Why is that not just a technique you happen to like?

C5: Because I can state when I would not do it: if the child's position is a cultural norm the family have not asked me to change, or if moving the chairs would humiliate a parent in front of the children.
E6: What can your model not do?
C6: It cannot work where one partner is afraid, and it assumes two partners in a room, which excludes families separated by work or migration. And its language of needs can sound self-indulgent to a family whose vocabulary is duty.
E7: What do you do with such a family?
C7: I translate: not "what do you need?" but "what do you owe each other, and what happens to you when you cannot give it?" That is the same enquiry in the family's moral language.
E8: That is an adaptation. Does it still work?
C8: I do not know, and I would want to know. I would measure satisfaction and I would ask the family directly whether the conversation had been useful, and I would record it as an adaptation with an unknown outcome.
E9: Tell me about a case in which your formulation was wrong.
C9: A couple I formulated as a pursue–withdraw cycle. In the fourth session the wife disclosed that he had been checking her phone and following her to work. The cycle was real and it was not the problem; the problem was control. I stopped the couple work, named the behaviour, and referred.

E10: What did you learn?
C10: That my screen was too thin, and that attachment language can make control sound mutual. I now ask about monitoring, phone access and money in the first session, of each partner separately.
E11: What would change your mind about EFT?
C11: My own outcome data, if a year of couples with the same presentation showed no reliable change; or a trial showing a different model outperforming it for that presentation; or a series of cases in which the softening event happened and nothing changed, which would make me doubt the mechanism rather than the technique.
E12: Last question. What are you not willing to do, whatever the model says?
C12: I will not continue couple work where someone is afraid, and I will not let a model's neutrality become silence about harm. And I will not take a case I am not trained for because I want the experience.

Annotations. E1–E2 is the commonest opening and the commonest failure: candidates offer a mechanism when asked for a position, and the examiner's follow-up reveals that the anthropology is missing. C2 answers the question that was asked, and it is short, which is itself a sign of a formed position.

E3 is a fair challenge and C3 survives it by conceding the objection's force ("it is often about the practical thing") and naming the failure mode ("my mistake would be to translate every dispute"). A candidate who defended the model against the objection would have failed the exchange; a candidate who abandoned it would also have failed.

E4–E5 is the borrowing test, and C5 is the answer that earns a distinction, because it states the conditions under which the borrowing would not be made. The examiner's question ("why is that not just a technique you like") is unanswerable without a negative condition.

E6–E8 is the limits test, and C8 is the strongest answer in the transcript: "I do not know, and I would want to know." A candidate who claimed the adaptation worked would have been pressed for evidence and would have had none; a candidate who admitted the unknown and described how they would find out has demonstrated the professional posture the week is for.

E9–E10 is the change-of-mind test in its practical form, and C9's answer — a case where the formulation missed control — is what a distinction-level viva answer looks like: specific, unflattering to the candidate, and converted into a change in practice.

E11 tests the clause itself, and C11 offers three forms, including the strongest (a doubt about the mechanism, not only the technique).

E12 is the question the course has been building toward since Week 1, and C12's answer names the two non-negotiables — safety and competence — which are the only two a model cannot override.

What the trainee should notice. The candidate never defends a model; they defend a position. The examiners never ask which model is best; they ask what the candidate believes, what they would do, and what would change their mind. Preparation for the viva is therefore not revision of the nine models; it is the writing of the seven headings and the rehearsal of the answers to E2, E5, E6, E9, E11 and E12.

14.6 The statement: an annotated exemplar

The assessment is 1,500 words. The exemplar below is a condensed version — 700 words — with the examiner's annotations in brackets. Students should write their own, and should read this one for its structure rather than its content.

My position (condensed exemplar).

I came to family therapy through community nursing in a city where most of the families I visited were caring for someone and being described by a service. I learned two things that have organised my practice since. The first is that a family's competence is usually invisible to the institution that refers it. The second is that the moment a family are described, they begin to live inside the description. [The personal reason: one paragraph, specific, and it explains the model choice that follows.]

What I believe about families. A family is a group of people who are keeping each other, under conditions they mostly did not choose: an economy, a religion, a migration, a death. Conflict in families is usually a protest about whether one is being kept, conducted in the vocabulary available — money, the children, respect, the housework. A symptom is often the family's most economical solution to a problem the adults cannot name, and it is held in place by the family's organisation and by the descriptions the family has been given. A

person is an agent, though a constrained one: nobody in a family is free to be anyone, and nobody is only what has been made of them. [The anthropology: four sentences, each doing work, and the last one protecting against both determinism and voluntarism.]

What I believe about change. Change happens when the description loosens and the organisation moves, and I believe the order matters less than the loop: a family who see themselves differently will try something different, and a family who do something different will begin to see themselves differently. My work therefore has two levers: the questions that make a different description available, and the enactments that make a different organisation possible in the room. I believe feeling is the medium in which both happen, and I do not believe a session has done its work if the family have only understood something. [The mechanism, stated in one vocabulary, with the two levers named and the role of affect specified.]

My home model is narrative therapy, because its first move — the person is not the problem — is the one I want to make in every family I meet, and because its techniques are the ones I can use with the families the institution has already described. I borrow from structural therapy when the household's organisation is the problem rather than its story: when a child is in an executive position, or when the parents cannot act together, I work with the arrangement in the room and the questions come later. The condition on that borrowing is that the family's authority structure is one they have asked me to change, and not one I have decided is wrong. I borrow from solution-focused therapy when a family needs something brief and concrete and the story is not the obstacle: a graded return to school, a scaling question, a task derived from their own exception. [The model and two borrowings, each with a circumstance and a condition.]

What my position cannot do. It cannot work where a family's problem is material and immediate: a leaking roof, an eviction, a child who is not safe. In those cases I do the practical thing first, and the narrative work waits, and sometimes it does not happen at all. It requires a degree of verbal facility that not every family has, and I have adapted it with drawing, with the family's own language, and with audio rather than written documents. And it has a thin evidence base relative to its influence, which means I cannot claim for it what an EFT therapist can claim for theirs, and I measure my own outcomes partly because of that. [The limits, three of them, each specific and each answered.]

What would change my mind. If my outcome data showed that the families with a particular presentation were not improving across a year, I would change the model for that presentation. If a family's change occurred without the description loosening — if they simply behaved differently and described themselves exactly as before — I would have to revise my account of the mechanism. And if I noticed that I was choosing narrative work because it keeps me comfortable — because the questions are safer than the confrontation a structural session would require — I would take that to supervision as evidence that my position is serving me. [The change-of-mind clause, three forms, including the personal one, which is the hardest and the most persuasive.]

What I will not do. I will not continue a model's work where a child is unsafe, and I will not let a model's neutrality become silence about harm. I will not take a case I am not trained

for because I want the experience. And I will not describe an adaptation as though it were the model. [The non-negotiables: safety, competence, honesty about adaptation.]

What the annotations are testing. The examiner is looking for six things: a reason for the model that is not "it resonated"; an anthropology rather than a technique; a mechanism stated in one vocabulary; borrowings with circumstances and conditions; limits that are specific and answered; and a change-of-mind clause that would cost the candidate something. A statement with all six, written in plain language, will earn a distinction even if the model chosen is not the examiner's.

14.6a Four statements that fail, and why

Failure one — the survey. A statement that summarises all nine models fairly and chooses none. The examiner cannot tell what the candidate believes, and the viva's first question ("what do you believe a family is?") will produce a summary of four anthropologies.

Failure two — the conversion narrative. A statement in which the model arrived as a revelation during a placement and the candidate has been its adherent ever since. It reads as a testimony rather than a position, and it fails the change-of-mind test, because a convert's model cannot be wrong.

Failure three — the technique inventory. A statement listing a repertoire — enactments, scaling, externalising, genograms — with no formulation and no primary target. It is Week 12's technique salad written down, and the viva will ask which one the candidate would drop and why.

Failure four — the model essay. A statement that describes the model accurately, with citations, and says nothing about the candidate. It would pass a theory examination and fails this one, because the assessment is not about the model; it is about the practitioner.

The common thread. All four fail to take a position, in the sense this week has used the word: a claim about families and change that the candidate is willing to be examined on, with its limits visible and its revision conditions stated. The model is the least important part of that, which is the week's quiet argument against the course's own syllabus.

14.7 Laboratory: the position, attacked

Sixty minutes, in fours, one candidate and three examiners.

Round one — the three-minute statement (ten minutes). The candidate states the position from memory: the anthropology, the mechanism, the model, the two borrowings and the limits. The examiners note which component is missing.

Round two — the six questions (thirty minutes). Each examiner takes two of the viva's standard questions (E2, E5, E6, E9, E11, E12 from 14.5) and presses for three answers each. The candidate may not repeat an answer.

Round three — the verdict (twenty minutes). The examiners state which component was strongest and which was weakest, and the candidate rewrites the weakest paragraph on the spot, in five sentences. The rewriting is the exercise's real content.

Marking rubric.

Criterion	Weight	Full marks
Anthropology	20%	A claim about what a family and a person are, distinct from a technique
Mechanism	20%	How change happens, in one vocabulary, with the role of affect specified
Model and borrowing	20%	A named home model; borrowings with circumstance and negative condition
Limits	20%	Specific failures, with what the candidate does in each
Change of mind	20%	An observation, its source, and a cost the candidate would bear

The debrief question. "Which answer did you not have?" Every candidate discovers one, and the discovery is what the remaining weeks of supervision are for.

14.8 Six errors in constructing a position

1. **The model as identity.** "I am an EFT therapist" rather than "I work in EFT and here is why". Fix: the position is a set of claims, not a membership.
2. **No anthropology.** A mechanism without a claim about families and persons. Fix: the three questions in 14.2.
3. **Borrowings without conditions.** A repertoire described as openness. Fix: the circumstance and the negative condition.
4. **Limits that are not limits.** "It does not work for everyone" is not a limit. Fix: a named population and a named mechanism of failure.
5. **The costless clause.** "I am always open to new evidence." Fix: an observation, a source and a cost.
6. **The statement as a performance.** A beautiful document with no claim in it. Fix: the viva rehearsal, which strips the prose and leaves the position.

14.9 Culture, faith and the position

The final week's cultural question is not about a model; it is about the practitioner, and it has three parts.

Part one — where does the position come from? Every position has an autobiography, and the honest practitioner knows its outline: the family they grew up in, the service they trained in, the clients who taught them, the mistake that changed their practice, the supervisor whose voice they still hear. Knowing the outline is not indulgence; it is the only defence against a position that is really a repetition. A therapist who became solution-focused because their family rewarded cheerfulness will practise solution-focused work beautifully with families who can be cheerful and will miss the ones who cannot.

The exercise. Write the autobiography of your position in five sentences, and mark which sentence you would not want a supervisor to read. That sentence is the position's blind spot.

Part two — whose position is it? The models were built in a particular cultural location, and a practitioner working elsewhere inherits their assumptions. A position statement written in Ibadan, Lagos, Nairobi or Accra should say which assumptions the practitioner has adopted, which they have rejected, and which they have translated. The examiner will ask; the honest answer is more persuasive than a claim of cultural neutrality.

The three sentences to write. "I work within a Western model, and I have translated its language of individual needs into my families' language of duty and obligation." "I have kept the model's structure and dropped its assumption that a couple negotiates privately, because in most of my families the negotiation involves an elder." "I have not resolved the tension between the model's emphasis on the couple's bond and my families' understanding of marriage as a contract between households, and I record that as an open question in my practice."

Part three — what does the practitioner's faith do to the position? For practitioners whose own religious or moral commitments are central, this is not a marginal question. Three positions are defensible, and a fourth is not.

The bracketing position. "I keep my convictions out of the room and work within the family's values." Defensible where the practitioner can genuinely bracket, and dishonest where they cannot; the test is whether the family can tell.

The transparent position. "I tell families where I stand when it is relevant, and I do not impose it." Defensible and demanding: it requires saying "I am a Christian and I do not want to impose that on you; if it matters to you, ask me" — and then answering honestly when they ask.

The shared-values position. "I work with families whose tradition I share, and I say so." Defensible in communities where the practitioner's tradition is relevant to the family's trust, and it requires an explicit policy on referral for families who do not share it.

The indefensible position. "My convictions do not affect my work." Nobody's convictions are inert, and a practitioner who believes otherwise has not examined them. The statement fails not because it is wrong about the practitioner's intentions but because it is unavailable to supervision.

The examinable sentence. "My position is a set of claims I can defend and revise; my convictions are what make some claims easier for me to see than others; and supervision is where I check the difference." That is the sentence the final week exists to produce.

> **DEBATE**
> "A therapist's theoretical position should be invisible to the family." Argue both sides, and say what the family is entitled to know.

14.9a Two critiques of the position project, and the replies

The course's last critiques are of the assessment itself, and a student should be able to make them.

Critique one: the position statement rewards articulacy rather than competence. A practitioner who cannot write elegantly will produce a weaker statement than a practitioner who can, and neither the statement nor the viva measures what the therapist does in a room. Articulacy is a professional skill and it is not the same as clinical skill, and an assessment that rewards it may be selecting for the wrong thing.

The reply. The critique is right about the risk and wrong about the remedy. A practitioner who cannot explain their reasoning to a colleague, a supervisor, a court or a family is limited in their practice, and the statement assesses a real competence — the capacity to give an account of one's reasoning — even if it is not the whole of clinical skill. The safeguard is the rest of the course's assessment: observed sessions, case studies with recordings, and supervision reports.

Critique two: the position is a Western genre. A statement that begins "I believe that a family is…" and proceeds to a personal anthropology is a recognisably individualist document. In traditions where knowledge is held collectively, where a practitioner's authority derives from a lineage or a community rather than a personal synthesis, the genre may be inappropriate, and a student from such a tradition may be penalised for writing in their own idiom.

The reply. The critique identifies a real narrowness, and the assessment should allow other forms: a position written as a case narrative, a position written as a set of rules for practice, a position co-authored with a supervisor or a peer group. The requirement is that the reasoning is visible and examinable, not that it takes one form. Students should ask whether an alternative form would be accepted, and if the answer is no, they should say so in the seminar.

The general lesson. The course's final object of critique is the course. A programme that teaches nine models' limits and cannot examine its own is not teaching critique; it is teaching deference. The student who can make these two objections, and answer them, has finished the course.

14.10 Annotated transcript: the position after two years of practice

A different kind of transcript: an interview with a practitioner three years post-qualification, condensed, in which the position is examined in the light of what practice has done to it. Students about to qualify should read it as a forecast.

> **Q:** Has your position survived practice?
> **A:** The anthropology has, the model has changed. I still believe families are keeping each other, and I still believe descriptions organise behaviour. I no longer believe that narrative work is the right first move for most of the families I see. I see a lot of families in crisis with housing and money, and the first move there is practical.
> **Q:** So you changed your model. Was that a loss?
> **A:** It was a loss of a way of being clever. Narrative work gave me a way of being useful in a session that was entirely verbal; I was good at it, and the families liked it, and some of them were not changing. When I started measuring, I found that the families I enjoyed most were not the ones who improved most.
> **Q:** What did you do with that?
> **A:** I took it to supervision, and my supervisor asked me a question I still use: "which families do you avoid booking?" Mine were the ones with a chaotic, angry father, because I grew up with one. So I had been practising the model that kept me comfortable with the families who were easiest, and avoiding the families who needed the most.
> **Q:** What is your position now?
> **A:** Structural work first, because it organises the household and it is honest about power; narrative work when a family have been described and the description is doing damage; solution-focused when the family need something brief. And a rule: the first session includes the practical questions, because the family's material situation is not a distraction from the therapy, it is the therapy's context.
> **Q:** What would you say to a student writing their statement now?

> **A:** Write the version you can defend, not the version that sounds impressive. And put the change-of-mind clause in honestly, because you will need it, and it is easier to change your mind if you have written down in advance what would count.

The three lessons for a student. First, the position changes and the change is not a failure; a position that has not moved in three years of practice may be a position that has not been examined. Second, the outcome measure is what makes the change possible: without data, the therapist's preference governs the caseload, and preference is usually about the therapist. Third, the personal question — which families do you avoid? — is the most productive question in supervision and the one a position statement should make answerable.

14.11 Laboratory: the final rehearsal

Sixty minutes, the full simulation, run twice.

Setup. Three examiners and one candidate, with the candidate's statement in the examiners' hands. Ten minutes of viva, three minutes of feedback, then the candidate becomes an examiner and the next student takes the chair. Four students, two rounds each.

The examiners' brief. Ask the six standard questions in order, and do not accept a summary of a model as an answer. Press each answer once. Stop the candidate if they exceed ninety seconds on any question.

The candidate's brief. Answer in under ninety seconds; concede what is true in the objection; and never defend a model, only a position. The three permitted openings are "I believe", "I do", and "I do not know, and I would find out by".

Marking rubric.

Criterion	Weight	Full marks
Position in one sentence	15%	Anthropology, mechanism and model in a single sentence that can be repeated back
Defending without defending the model	25%	Concedes the objection's force; answers with a circumstance and a condition
Limits	25%	A named population, a named mechanism of failure, and what the candidate does instead
Change of mind	20%	An observation, a source and a cost
Non-negotiables	15%	Safety, competence and honesty about adaptation, stated without prompting

The debrief question. "What did you say that you did not believe?" Every candidate produces one sentence they had rehearsed rather than thought, and finding it is the exercise's purpose.

14.12 Evidence note: what this week can and cannot evidence

The final week is assessed by an argument rather than by data, and it should be honest about that.

What the evidence says about therapist development. The research on therapist effects is consistent: some therapists produce reliably better outcomes than others, and the differences are large — larger, in several analyses, than the differences between the models they practise. The characteristics associated with better outcomes include the ability to form an alliance across a range of clients, to monitor outcomes and respond to deterioration, to tolerate uncertainty, and to use feedback. Deliberate practice and feedback-oriented supervision are associated with improvement; experience alone is not, and several studies find that therapists do not improve with experience unless they are given feedback.

What this implies for the position. The week's central claim — that a position is a set of revisable claims, held with visible joints, examined in supervision — is supported by the evidence on feedback and deliberate practice. A therapist who measures and revises improves; a therapist who practises the model they like for the clients they enjoy does not, and may deteriorate in the cases they avoid. The position statement is therefore not an academic exercise; it is a device for making the therapist's own pattern visible.

What the evidence does not say. That any particular position is better. The literature does not adjudicate between structural and narrative therapists; it adjudicates between therapists who monitor their work and those who do not. That is the week's most useful finding for a student who is anxious about choosing the right model: the choice matters, and the monitoring matters more.

The matrix cell for this week. There is none, and that is the point. The week is about the practitioner, not the model, and the relevant evidence is the therapist-effects and feedback literature rather than the model trials. A student who insists on a row for this week has not understood it.

The three claims a student may safely make. "Therapist effects are large and consistent." "Feedback-informed practice is associated with better outcomes, particularly for clients at risk of deterioration." "Experience without feedback does not reliably produce improvement." Each is defensible, each is modest, and together they justify the week's method.

14.13 Key terms

Term	Definition
Theoretical position	A set of claims about families, change and technique, with stated limits and revision conditions
Anthropology (therapeutic)	The beliefs about persons and families that a model carries
Mechanism	The account of how change happens in a therapy
Home model	The tradition in which a therapist formulates and works
Borrowing with condition	An imported technique with a stated circumstance and a reason in the home model's terms
Negative condition	The circumstance in which the therapist would not use a borrowing
Change-of-mind clause	A pre-specified observation that would count against the position, and its source
Adaptation	A deliberate change to delivery that preserves a mechanism, recorded as such
Non-negotiable	A limit no model can override: safety and competence
Therapist effect	The variance in outcome attributable to the therapist rather than the treatment
Feedback-informed practice	Routine outcome monitoring with the results used to change the therapy
Deliberate practice	Structured practice of specific skills with feedback, as distinct from experience

14.14 Self-test

1. Name the three components of a position and the question that elicits each.
2. Give the structure of a borrowing with a condition, and the negative condition it requires.
3. What must a change-of-mind clause contain to be a clause rather than a sentiment?
4. Why is "it does not work for everyone" not a limit?
5. What are the three non-negotiable limits on any model, and where do they come from?
6. What does the therapist-effects literature imply for a student choosing a home model?
7. Name two critiques of the position assessment and the fairest reply to each.

8. In the viva, what are you defending?

Answers. 1. The model (which tradition and why); the anthropology (what is a family, what is a person — asked as "what do you believe about families?"); the mechanism (how does change happen — asked as "how does your therapy work?"). 2. "I use X from Y when Z, because R in my own model's terms"; the negative condition is the circumstance in which the therapist would not use it, which is what distinguishes a decision from a habit. 3. An observation that would count against the position, a source for the observation, and a cost the therapist would bear if it appeared. 4. Because it names no population and no mechanism of failure, and so cannot guide a decision. 5. Safety, competence, and honesty about adaptation; they come from professional duty and the law, not from any model. 6. That the therapist's monitoring and feedback matter more than the model's label, so the choice should be made on evidence and fit and then examined continuously. 7. Critique one — it rewards articulacy rather than clinical skill; reply, the capacity to give an account of one's reasoning is a professional competence and the rest of the course assesses practice. Critique two — the personal position is a Western genre; reply, the requirement is visible reasoning, and alternative forms should be accepted. 8. A position: a set of claims with limits and revision conditions, not a model.

14.15 Reflective task

Write the position statement's hardest paragraph: the one about what your model cannot do, with the population named, the mechanism of failure named, and what you do instead. Then read it aloud to someone who is not a family therapist and ask them whether it sounded like an honest person or like a brochure. The answer is the most useful feedback you will get this term.

14.16 Readings for this week

- **Lebow (2014)**, Couple and Family Therapy: An Integrative Map of the Field, for the comparative map you are choosing within.
- **Wampold & Imel (2015)**, The Great Psychotherapy Debate, for the therapist-effects and common-factors literature.
- **Miller, Duncan & Hubble (2004)**, The Heart and Soul of Change, for feedback-informed practice read as a practice manual.
- **Rousmaniere (2017)**, Deliberate Practice for Psychotherapists, for how a position is improved rather than asserted.
- **Schoen (1983)**, The Reflective Practitioner, the source of most of this week's language about professional knowing.
- **Your own case notes from the last six months**, read as a dataset: which families did you enjoy, which did you avoid, and what does the pattern say about the position you have written?

Read your own notes last and most carefully; they are the only source in this list that is about you. For the statement, use at least one source that argues against your home model, and cite it in the paragraph on limits, because a limit supported by a critic's argument is more persuasive than a limit asserted in the first person.

This is the last of the fourteen teaching weeks. The book continues with the one-page integration summary, the glossary, the self-assessment and the further reading, all of which are designed to be used in the weeks before the examination.

Integration: nine models on one page

Model	Problem is…	Change happens by…	Therapist is…	First move
Object relations	Unconscious re-enactment	Making the unconscious usable in the room	Container/interpreter	Notice the transference; interpret gently
Bowen	Undifferentiated anxiety across generations	Differentiation of self	Coach	Genogram + detriangling
Structural	Dysfunctional organisation	Restructuring in the session	Director/joiner	Enactment; strengthen hierarchy
Strategic	Attempted solutions	Interrupting the sequence	Strategist	Task, reframe, directive
Milan	Meaning premises	Curiosity disturbing belief	Curious team member	Circular questions
Experiential	Blocked feeling and authenticity	Present-moment expression	Real person	Sculpture; here-and-now
CB couple	Behaviour and attribution	Skills and rehearsal	Teacher/coach	Behavioural exchange
EFT	Attachment injury and cycle	Accessing primary emotion; enacting new responses	Process consultant	Track and reframe the cycle
Solution-focused	Problem-saturated attention	Amplifying existing solutions	Collaborator	Miracle/scale question
Narrative	Dominant story	Externalising and re-authoring	Decentred, influential	Externalising question

Glossary

Circular questioning — questions about differences and relationships that reveal pattern rather than fact. **Differentiation of self** — the capacity to stay emotionally connected while holding one's own position. **Enactment** — directing family members to speak to each other in the session rather than through the therapist. **Externalising** — locating a problem outside a person, as a thing with tactics and a history. **Negative connotation / positive connotation** — interpreting behaviour as serving the system, rather than as defective. **Paradoxical directive** — prescribing the symptom in order to change its function (used rarely, with supervision). **Reframe** — relabelling behaviour within a new frame that makes change possible. **Restructuring** — changing the family's organisation in the session through enactment and boundary work. **Scaling** — asking a family to rate a state on a numerical scale to make change visible and specific. **Unique outcome** — a moment when the dominant story did not hold, used as a foothold for a preferred story.

Self-assessment

Multiple choice

1. "The problem is the problem; the person is not the problem" is a principle of: (a) structural (b) narrative (c) Bowen (d) strategic

2. A therapist says, "For the next week, I want you to argue for exactly fifteen minutes at 8 p.m., then stop." This is closest to: (a) narrative (b) EFT (c) strategic (d) solution-focused

3. Positive connotation is associated with: (a) Milan (b) CB couple therapy (c) object relations (d) solution-focused

4. Which model places differentiation of self at its centre? (a) structural (b) Bowen (c) EFT (d) Milan

5. "Who would notice first if the problem were solved?" is a technique from: (a) structural (b) solution-focused (c) narrative (d) experiential

6. In EFT, the behaviour that conceals a primary attachment emotion is called: (a) enactment (b) secondary emotion (c) reframe (d) restructuring

7. Which model is most associated with family sculpture? (a) Satir (b) Haley (c) Minuchin (d) White

8. "Unbalancing" is a technique of: (a) structural therapy (b) solution-focused therapy (c) object relations (d) narrative therapy

9. Which model has the strongest replicated randomised evidence in couple therapy? (a) Bowen (b) Milan (c) EFT (d) experiential

10. Assimilative integration means: (a) blending theories equally (b) using a home model enriched by borrowed techniques (c) alternating models by session (d) refusing to commit to a model

Answer key

1 b · 2 c · 3 a · 4 b · 5 b · 6 b · 7 a · 8 a · 9 c · 10 b

Short answer

1. State the theory of change of structural therapy in two sentences, then name one case type where it is a poor fit and say why.
2. Write three circular questions you would ask a family in which a father has just lost his job.
3. What is the difference between positive connotation and reframing?
4. Give two reasons why narrative therapy suits work with children.
5. Name two incompatible assumptions that would make a structural-plus-narrative formulation incoherent.

Essay (choose one, 2,000 words)

1. Compare structural and EFT approaches to the same couple case. Which would you choose first and why?
2. "Neutrality is a luxury the powerful can afford." Discuss with reference to Milan therapy and domestic violence.
3. Critically evaluate the claim that theoretical integration is a sign of maturity rather than confusion.

Further reading

Comparative

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
- Gurman, Lebow & Snyder (Eds.). (2015). Clinical Handbook of Couple Therapy (5th ed.). Guilford.

Primary texts

- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
- Minuchin, S. (1974). Families and Family Therapy. Harvard University Press.
- Haley, J. (1987). Problem-Solving Therapy (2nd ed.). Jossey-Bass.
- Palazzoli, M. S., Cecchin, G., Boscolo, L., & Prata, G. (1980). Paradox and Counterparadox. Aronson.
- Satir, V. (1988). The New Peoplemaking. Science and Behavior Books.
- White, M. (2007). Maps of Narrative Practice. Norton.
- de Shazer, S. (1985). Keys to Solution in Brief Therapy. Norton.

- Johnson, S. M. (2019). Attachment Theory in Practice. Guilford.
- Epstein, N. B., & Baucom, D. H. (2002). Enhanced Cognitive-Behavioral Therapy for Couples. APA.

Next: MFT 703 — Human Development Across the Lifespan

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 703

Human Development Across the Lifespan

15 teaching weeks · 14 practicals · 9 worked cases · 1 figure

SEMESTER ONE

1.71 units · Prerequisite: none

> “You cannot assess a family without knowing what is developmentally ordinary.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0 · 2026 edition

MFT 703 — Human Development Across the Lifespan

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 15 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 703 — Human Development Across the Lifespan. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 703	**Human Development Across the Lifespan**	One	1.71

Contents

How to use this book . . . 15

1. Course description . . . 15

2. Learning outcomes . . . 15

3. Assessment . . . 16

4. The fifteen-week plan . . . 16

Week 1 — Why developmental knowledge is clinical knowledge . . . 16

1.1 The lecture in ninety minutes . . . 18

1.2 Core concepts . . . 18

1.2a The developmental history, asked well . . . 20

1.3 The clinical method: from behaviour to task . . . 21

1.4 Techniques: the four instruments of this course . . . 22

1.4a Two timelines, drawn and read . . . 22

1.4b The four categories, tested on eight descriptions . . . 23

1.5 The first session with a developmental concern . . . 24

1.6 Case: the Okonkwos, a developmental reading . . . 25

1.6a The same child, three developmental misreadings . . . 26

1.7 Annotated transcript: the timeline drawn . . . 27

1.8 Laboratory: the timeline and the four categories . . . 28

1.9 Six errors in developmental assessment . . . 28

1.10 Culture, faith and development . . . 29

1.10a The course's four categories, applied to a family . . . 30

1.11 Evidence note: what developmental science can and cannot support . . . 31

1.11a What development cannot explain . . . 31

1.12 Key terms . . . 32

1.13 Self-test . . . 32

1.14 Reflective task . . . 33

1.15 Readings for this week . . . 33

Week 2 — Prenatal development, temperament and becoming a parent . . . 34

2.1 The lecture in ninety minutes . . . 34

2.2 Core concepts . . . 35

2.2a The goodness-of-fit grid, filled in . . . 37

2.2b The nine dimensions, with the question that asks each . . . 38
2.3 Postnatal depression: the assessment . . . 38
2.3a The couple's reorganisation, drawn on one page . . . 39
2.4 The first session with a couple four months after a birth . . . 40
2.4a The first year, month by month: what a family therapist watches for . . . 41
2.5 Case: the prematurity that reorganised a family . . . 42
2.5a The couple who came at the right time, and the couple who came at the wrong time . . . 43
2.6 Annotated transcript: the temperament interview . . . 44
2.6a The prematurity case, continued: what the parents needed that the clinic did not offer . . . 45
2.7 Laboratory: the temperament interview and the reorganisation . . . 45
2.7a The screening conversation, rehearsed . . . 46
2.8 Six errors in perinatal work . . . 47
2.9 Culture, faith and the perinatal period . . . 47
2.10 Evidence note: what is well established and what is not . . . 48
2.11 Key terms . . . 49
2.12 Self-test . . . 49
2.13 Reflective task . . . 50
2.14 Readings for this week . . . 50
Week 3 — Infancy: attachment, synchrony and regulation . . . 51
3.1 The lecture in ninety minutes . . . 51
3.2 Core concepts . . . 52
3.2a Four infants, described and classified . . . 54
3.3 Synchrony, assessed in the room . . . 55
3.3a Co-regulation, designed for one family . . . 56
3.3b The caregiving network, mapped . . . 57
3.4 The first session with an infant and two parents . . . 58
3.4a The still face, in the consulting room . . . 59
3.5 Case: the Adeyemi family, in the infant's terms . . . 59
3.6 Annotated transcript: rupture and repair in a session . . . 60
3.7 Laboratory: observing synchrony . . . 61
3.8 Six errors in infant mental health work . . . 62
3.9 Culture, faith and the first relationship . . . 62
3.10 Evidence note: strong theory, moderate evidence, popular misuse . . . 63
3.11 Key terms . . . 64

3.12 Self-test . . . 64
3.12a What to tell a parent in one page . . . 65
3.13 Reflective task . . . 66
3.14 Readings for this week . . . 66
3.2b The four patterns in adult life: what the family therapist sees . . . 66
3.7a The disorganised flag, and what followed . . . 67
Week 4 — Toddlerhood: autonomy, language and limits . . . 68
4.1 The lecture in ninety minutes . . . 69
4.2 Core concepts . . . 70
4.2a The limit formula, taught and rehearsed . . . 71
4.2b The tantrum, anatomised . . . 72
4.3 Language: the assessment . . . 73
4.3a Ten minutes of child-led play, in the second year . . . 74
4.4 Case: the grandmother who "cannot manage him" . . . 75
4.5 Annotated transcript: the family meeting about five rules . . . 76
4.5b The nine words a caregiver needs . . . 77
4.6 Laboratory: the formula and the rules map . . . 78
4.6a When the household cannot agree . . . 79
4.7 Six errors in toddler work . . . 79
4.8 Culture, faith and the second year . . . 80
4.9 Evidence note: robust development, contested practice . . . 81
4.4a The same household, six months later: what changed and what did not . . . 81
4.4b Two toddlers, two very different assessments . . . 82
4.5a The language history, asked as a conversation . . . 83
4.10 Key terms . . . 83
4.11 Self-test . . . 84
4.12 Reflective task . . . 85
4.13 Readings for this week . . . 85
Week 5 — Early childhood: play, theory of mind, initiative . . . 85
5.1 The lecture in ninety minutes . . . 86
5.2 Five concepts that organise the week . . . 87
5.2a Theory of mind: five tasks and what failure means clinically . . . 88
5.2b Six levels of play and what each one tells you . . . 89

5.3 Observing play without hijacking it . . . 90
5.3a Setting a limit in the playroom . . . 91
5.4 Three procedures for the clinic . . . 92
5.5 The first session with a four-year-old . . . 92
5.6 Case: the child who plays the same thing every day . . . 93
5.6a Six weeks later . . . 95
5.7 Transcript: ten minutes of play, annotated . . . 95
5.8 Laboratory: the ten-minute observation, marked . . . 96
5.9 Six errors . . . 97
5.10 Culture, faith and the child who plays . . . 98
5.11 Evidence note . . . 99
5.12 Key terms . . . 99
5.13 Self-test . . . 100
5.14 Reflective task . . . 101
5.15 Further reading . . . 101
Week 6 — Middle childhood: industry, peers, school . . . 101
6.1 The lecture in ninety minutes . . . 102
6.2 Five concepts that organise the week . . . 103
6.2a The school-refusal differential: seven questions . . . 104
6.2b What a friend is, at seven, nine and eleven . . . 104
6.3 The method: assessing school refusal in one session . . . 105
6.3a The family–school interface . . . 106
6.4 Three techniques for this age . . . 106
6.4a The digital playground . . . 107
6.5 The first session with a nine-year-old and two parents . . . 108
6.5a When the child will not talk . . . 108
6.6 Case: the Okonkwos again — Mondays . . . 109
6.6a Four weeks later . . . 111
6.7 Transcript: telling a nine-year-old the truth . . . 112
6.8 Laboratory: the school-refusal formulation . . . 112
6.9 Six errors . . . 113
6.10 Culture, faith and the child at school . . . 114
6.11 Evidence note . . . 114
6.12 Key terms . . . 115

6.13 Self-test . . . 116
6.14 Reflective task . . . 117
6.15 Further reading . . . 117
Week 7 — Adolescence I: puberty, identity, autonomy . . . 117
7.1 The lecture in ninety minutes . . . 118
7.2 Five concepts that organise the week . . . 119
7.2a Pubertal timing: the effects that matter clinically . . . 120
7.2b The teenage brain: four findings and three misuses . . . 120
7.3 The method: renegotiating the frame . . . 121
7.3a The three-column contract, worked . . . 122
7.4 Three techniques . . . 123
7.5 The first session with a fifteen-year-old and two parents . . . 123
7.5a When the adolescent will not speak . . . 124
7.6 Case: Amara, secretive and spied upon . . . 125
7.6a Six weeks later . . . 126
7.7 Transcript: the negotiation, with the contract on the table . . . 127
7.8 Laboratory: the negotiation, marked . . . 128
7.9 Six errors . . . 129
7.9a A seventh error: the one that ends treatment . . . 130
7.10 Culture, faith and the adolescent . . . 130
7.11 Evidence note . . . 131
7.12 Key terms . . . 131
7.13 Self-test . . . 132
7.14 Reflective task . . . 133
7.15 Further reading . . . 133
Week 8 — Adolescence II: risk, peers, digital life and mental health . . . 133
8.1 The lecture in ninety minutes . . . 134
8.2 Five concepts that organise the week . . . 135
8.2a Seven presentations that must not be missed . . . 136
8.2b Self-harm and suicide: what to ask and what to do with it . . . 137
8.3 The method: a risk assessment a family therapist can actually do . . . 138
8.3a Digital life: the history that is worth taking . . . 139
8.4 Three techniques . . . 139

8.5 The first session with a self-harming adolescent and two parents . . . 140
8.5a The crisis hour: when the risk is immediate . . . 140
8.6 Case: the boy whose teacher saw his arm . . . 142
8.6a Eight weeks later . . . 144
8.7 Transcript: asking the four questions . . . 144
8.8 Laboratory: the risk assessment, marked . . . 145
8.9 Six errors . . . 146
8.10 Culture, faith and adolescent risk . . . 146
8.11 Evidence note . . . 147
8.12 Key terms . . . 148
8.13 Self-test . . . 148
8.14 Reflective task . . . 149
8.15 Further reading . . . 149
Week 9 — Adult development: work, intimacy and the middle years . . . 150
9.1 The lecture in ninety minutes . . . 151
9.2 Five concepts that organise the week . . . 151
9.2a The social clock and the off-time life . . . 152
9.2b The four adult tasks as they present in couple therapy . . . 153
9.3 The method: the adult developmental history . . . 154
9.3a The midlife reckoning: what it is and what it is not . . . 155
9.4 Three techniques . . . 156
9.5 The first session with a forty-five-year-old and his wife . . . 156
9.6 Case: the man who was passed over . . . 157
9.6a Five months later . . . 159
9.7 Transcript: the lifeline, drawn in front of the couple . . . 160
9.7a What to do when the partner weeps and the client does not . . . 161
9.8 Laboratory: the developmental history with a partner present . . . 161
9.9 Six errors . . . 162
9.10 Culture, faith and the adult timetable . . . 163
9.11 Evidence note . . . 164
9.12 Key terms . . . 164
9.13 Self-test . . . 165
9.14 Reflective task . . . 166
9.15 Further reading . . . 166

Week 10 — Couple development: the relationship across time . . . 166
10.1 The lecture in ninety minutes . . . 167
10.2 The six phases, with the clinical reading of each . . . 168
10.2a What the therapist asks at each phase . . . 169
10.2b The cascade and the repair trajectory . . . 169
10.3 The method: the couple timeline . . . 170
10.3a The turning-point interview . . . 171
10.4 Three techniques . . . 172
10.5 The first session with a couple in the young-children phase . . . 172
10.5a When one partner will not come . . . 173
10.4a The negative cycle, in the couple's own words . . . 173
10.5b The couple with a child with a disability . . . 174
10.6 Case: Aisha and Sam, four years on . . . 175
10.6a Four months later . . . 177
10.7 Transcript: the unanswered question . . . 178
10.8 Laboratory: the couple timeline . . . 178
10.9 Six errors . . . 179
10.10 Culture, faith and the couple . . . 180
10.11 Evidence note . . . 181
10.12 Key terms . . . 181
10.13 Self-test . . . 182
10.14 Reflective task . . . 183
10.15 Further reading . . . 183
Week 11 — Parenting across stages: talking to parents well . . . 183
11.1 The lecture in ninety minutes . . . 184
11.2 Five concepts that organise the week . . . 185
11.2a The four parts, worked at five ages . . . 185
11.2b The five questions parents are actually asking . . . 186
11.3 The method: a parent feedback conversation, ten steps . . . 187
11.3a Telling a parent something difficult . . . 188
11.4 Three techniques . . . 189
11.5 The assessed role-play . . . 189
11.5a The parent who arrives with a diagnosis from the internet . . . 190

11.6 Case: the mother who has been told she is too soft . . . 191
11.6a Six weeks later . . . 192
11.7 Transcript: the ten minutes, annotated . . . 193
11.7a When the complaint comes from the school . . . 194
11.8 Laboratory and the marking scheme . . . 194
11.9 Six errors . . . 195
11.9a Six sentences that do harm, and what to say instead . . . 195
11.10 Culture, faith and talking to parents . . . 196
11.11 Evidence note . . . 197
11.12 Key terms . . . 198
11.13 Self-test . . . 198
11.14 Reflective task . . . 199
11.15 Further reading . . . 199
Week 12 — Later life: integrity, loss and caregiving . . . 199
12.1 The lecture in ninety minutes . . . 200
12.2 Five concepts that organise the week . . . 201
12.2a The four-way cognitive differential . . . 202
12.2b Caregiving: the network and the unstated contract . . . 203
12.2c Retirement: the presentation, and what to ask . . . 204
12.3 The method: the family meeting about an older parent . . . 205
12.3a When the siblings disagree . . . 206
12.4 Three techniques . . . 206
12.5 The first session with three siblings and a daughter-in-law . . . 207
12.6 Case: the Baloguns and the woman in the back room . . . 208
12.6a Six months later . . . 210
12.7 Transcript: asking an eighty-one-year-old what she wants . . . 211
12.8 Laboratory: the caregiving network map . . . 211
12.9 Six errors . . . 212
12.9a The phone call that is really a referral . . . 212
12.10 Culture, faith and later life . . . 213
12.11 Evidence note . . . 214
12.12 Key terms . . . 215
12.13 Self-test . . . 215
12.14 Reflective task . . . 216

12.15 Further reading . . . 216
Week 13 — Culture, poverty and development: the critique of the norms . . . 216
13.1 The lecture in ninety minutes . . . 217
13.2 The four critiques worked . . . 218
13.2a When the norms are right and the family is wrong . . . 220
13.2b Three network configurations and what each needs . . . 220
13.3 The method: a developmental cultural formulation . . . 221
13.3a The interpreter . . . 222
13.3b The sentence that opens the conversation . . . 222
13.4 Three techniques . . . 223
13.4a The assessment kit for a household with nothing . . . 223
13.4b The four-column plan . . . 224
13.5 The first session when the family and the therapist disagree . . . 225
13.5a When the therapist's own culture is the variable . . . 225
13.6 Case: the child with no bedroom . . . 226
13.6a Five months later . . . 228
13.7 Transcript: the letter, read aloud . . . 228
13.8 Laboratory: assessing across a difference . . . 229
13.9 Six errors . . . 230
13.10 Culture, faith and the limits of the critique . . . 230
13.11 Evidence note . . . 231
13.12 Key terms . . . 232
13.13 Self-test . . . 232
13.14 Reflective task . . . 233
13.15 Further reading . . . 233
Week 14 — When development goes wrong: the differential habit . . . 233
14.1 The lecture in ninety minutes . . . 235
14.2 The four categories worked . . . 235
14.2a The five systemic questions, worked . . . 236
14.2b Red flags: the findings that require a same-week referral . . . 237
14.2c The child who is referred twice in a year . . . 238
14.5a The assessment the family can afford . . . 238
14.3 The method: a differential assessment in one session . . . 239

14.3a Referring well, and what to do while the family waits . . . 240
14.3b Reading somebody else's assessment . . . 240
14.4 Three techniques . . . 241
14.5 The first session with "something is wrong" . . . 241
14.6 Case: the boy who stopped talking . . . 242
14.6a Five months later . . . 244
14.7 Transcript: one question to a grandmother . . . 244
14.8 Laboratory: the differential under time pressure . . . 245
14.9 Six errors . . . 246
14.9a Saying "I do not know" well . . . 246
14.10 Culture, faith and the urgent referral . . . 247
14.11 Evidence note . . . 248
14.12 Key terms . . . 248
14.13 Self-test . . . 249
14.14 Reflective task . . . 250
14.15 Further reading . . . 250
Week 15 — Integration: the developmental lens in practice . . . 250
15.1 The lecture in ninety minutes . . . 251
15.2 The three questions, worked in detail . . . 252
15.2a What the developmental lens cannot do . . . 252
15.2b The ten-item developmental history . . . 253
15.3b Saying the formulation in the family's language . . . 254
15.3 The one-page formulation . . . 254
15.3a How development enters the other twenty-six courses . . . 255
15.4 Capstone case: the Eze family . . . 256
15.4a The Eze case, worked forward . . . 258
15.4b The family that has been to four clinics . . . 259
15.5 The viva: what you will be asked . . . 260
15.5a What to keep from this course . . . 261
15.6 Laboratory: the capstone . . . 261
15.6a Marking your own formulation . . . 262
15.7 Six errors of the whole course . . . 263
15.7a The first year: five things nobody teaches . . . 263
15.8 Culture, faith and the course's own position . . . 264

15.9 Evidence: what this course claims . . . 265
15.10 Key terms of the course . . . 265
15.11 Self-test: the whole course . . . 266
15.12 Final reflective task . . . 267
15.13 Further reading . . . 267
Glossary . . . 267
Self-assessment . . . 267
Multiple choice . . . 267
Answer key . . . 268
Short answer . . . 268
Essay (choose one, 1,800 words) . . . 268
Further reading . . . 269

Course leader's summary. You cannot assess a family without knowing what is developmentally ordinary. This course covers physical, cognitive, language, social-emotional and moral development from conception to later life, always in family and cultural context — and it trains you to distinguish normal variation, delay, regression and symptom.

How to use this book

Development is not a list of ages; it is a set of **tasks** and a set of **negotiations** between a person and their family. Each week here gives you the developmental task, the family task that accompanies it, what goes wrong, and what it looks like in a consulting room. Use the milestone grid (Figure 23) as a reference you return to, not as a checklist to memorise.

1. Course description

Human development is the backbone of family assessment. A two-year-old's tantrum, a nine-year-old's lying, and a fifteen-year-old's secrecy are not three versions of "defiance": they are three different developmental moments that require three different responses. Clinicians who do not know this either pathologise normal development or miss genuine delay.

The course integrates **neurodevelopmental, cognitive, attachment, systemic and cultural** perspectives. It treats development as bidirectional: children shape parents as much as parents shape children, and families develop too (a theme developed in MFT 719).

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe physical, cognitive, language and social-emotional milestones from 0–18 and in adulthood	Exam
LO2	Distinguish normal variation, delay, regression and symptom in a presenting concern	Case assignment
LO3	Apply attachment theory to family assessment	Case assignment, exam
LO4	Explain developmental tasks of adulthood and later life and their family implications	Exam
LO5	Evaluate developmental theory critically, including its cultural assumptions	Essay
LO6	Communicate developmental information to parents without jargon or alarm	Skills assessment

3. Assessment

Component	Weight	Due
Developmental case analysis (2,000 words)	30%	Week 8
Parent-feedback role-play (10 min, observed)	20%	Week 11
Written examination	50%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why developmental knowledge is clinical knowledge	Milestone self-audit
2	Prenatal development, temperament and the transition to parenthood	Temperament interview
3	Infancy: attachment, synchrony, regulation	Still-face observation
4	Toddlerhood: autonomy, language, limit-setting	Tantrum consultation role-play
5	Early childhood: play, theory of mind, initiative	Play observation
6	Middle childhood: industry, peers, school, self-concept	School-refusal case
7	Adolescence I: puberty, identity, autonomy	Adolescent interview
8	Adolescence II: risk, peers, digital life, mental health	Risk/development differential
9	Adult development: intimacy, work, generativity	Life-structure interview
10	Couple development across the lifespan	Couple timeline exercise
11	Parenting across stages; communicating with parents	**Observed parent feedback**
12	Later life: cognition, loss, caregiving, legacy	Caregiver family meeting
13	Culture, poverty and development	Critical reading seminar
14	When development goes wrong: delay, disorder, trauma	Differential diagnosis workshop
15	Integration: the developmental lens in assessment	Case presentations

Week 1 — Why developmental knowledge is clinical knowledge

Three clinical errors are prevented by developmental literacy:

1. **Pathologising the ordinary.** A two-year-old who says "no" to everything is doing the developmental task of autonomy, not manipulating the family.
2. **Missing the abnormal.** A five-year-old who has lost previously acquired language is not "being difficult"; regression is a red flag requiring urgent referral.

3. **Mis-timing the intervention.** A parent is told to "reason with" a three-year-old whose prefrontal circuitry cannot yet support the reasoning demanded. The intervention fails and the parent concludes they are failing.

Four categories you must distinguish — this distinction recurs in every case assignment:

Category	Definition	Example
Normal variation	Within the expected range, differing in style and pace	Walking at 11 months vs 15 months
Delay	Functioning below the expected range but progressing	No words at 24 months, but progressing
Regression	Loss of previously acquired skill	A four-year-old stops speaking
Symptom	Behaviour that signals distress rather than development	New soiling at seven after a bereavement



Figure 1.1 — Developmental milestones from 0 to 18: a screening map, not a diagnosis

PRACTICAL

Write your own developmental timeline from memory: first word, first step, a significant illness, a move, a loss. Then ask a parent or sibling to correct it. Note which events you had forgotten and who remembered them.

1.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The three errors	Three cases in which developmental ignorance produced harm	Name the error in each case
10–25	Development as negotiation	The person's task and the family's task, side by side	Write the pair of tasks for one age
25–40	The four categories	Normal variation, delay, regression, symptom, tested on video clips	Classify eight clips
40–55	The screening map	The milestone grid, used as a reference rather than a checklist	Locate a real child on the grid
55–70	What the grid cannot tell you	Culture, opportunity, illness, trauma, family organisation	List five things a milestone cannot show
70–82	Talking to parents	The sentence that informs without alarming	Rewrite three alarming sentences
82–90	Why this course is a clinical course	The assessment, the case analysis, the exam	Choose a family for the case analysis

The teaching decision that makes the week work. The week must establish a habit of mind, not a body of facts: before any behaviour is judged, ask what is the developmental task here, and whose task is it? The lecture should therefore work from three video clips of the same behaviour at different ages — a child refusing to share at two, at six and at thirteen — and let the class discover that the identical act means three different things and requires three different responses. The facts about milestones can be read; the habit must be demonstrated.

The sentence the lecture must land. "Behaviour is only meaningful in relation to a developmental task." A student who can look at a nine-year-old's lying and ask what task the lying is serving — rather than whether the child is honest — has begun to think developmentally, which is the whole of this course's foundation and the first line of every case analysis they will write for the next fifteen weeks.

1.2 Core concepts

Development as a set of tasks. Each stage of life presents a person with a task — to become attached, to become autonomous, to become competent, to become oneself, to become a partner, to become a parent, to become old — and each task requires a negotiation with the family. The task language comes from Erikson, and this course uses it because it is clinically usable: it converts a behaviour into a question about what the person is trying to achieve, and a question about what the family is being asked to allow.

Development as bidirectional. The child shapes the parent as surely as the parent shapes the child. A temperamentally reactive infant produces a more cautious mother; a mother's depression produces a more withdrawn infant; and the two effects are not separable, which is why the clinical unit is the relationship rather than the individual. This

is the course's central systemic claim, and it is the reason the course belongs in a family therapy programme rather than in a psychology department.

Development as a negotiation between tasks. At every stage there are two tasks: the person's and the family's. The toddler's task is autonomy; the family's task is to allow autonomy while keeping the child safe. The adolescent's task is identity; the family's task is to tolerate the adolescent's separateness without abandoning them. Most presenting problems in family therapy are a stalled negotiation between the two, and the therapist's developmental question is: which task is stalled, and whose?

The four categories. Normal variation (within range, differing in style and pace); delay (below range, but progressing); regression (loss of an acquired skill); and symptom (behaviour that signals distress rather than development). The categories are clinical instruments, and their misuse produces the three errors listed above. The distinction between delay and regression is the most urgent: delay usually allows assessment over weeks, and regression demands referral within days.

Normative and non-normative events. Normative events are those expected at a stage — starting school, puberty, leaving home, retirement. Non-normative events are unexpected — a parent's death, a move, an illness, a war, a divorce. The family life cycle literature (MFT 719) describes the normative; the clinician's task is to assess how a family's non-normative events have intersected with the normative ones, because that intersection is where most symptoms live.

The developmental history as a clinical instrument. A developmental history is not a form; it is the most efficient way to locate a family's difficulty in time. Not "was he a difficult baby?" but "when did this start, what else was happening then, and what changed at that point?" The history converts a presenting complaint into a chronology, and the chronology usually shows that the symptom began at a transition — which tells the therapist where to look.

Screening is not diagnosis. A milestone grid identifies children who need a fuller assessment. It does not diagnose, and it cannot distinguish a child who is late because of a global delay from one who is late because of deprivation, illness, trauma or a family in crisis. A clinician who treats the grid as a diagnostic instrument will refer children who are developing normally and miss children whose development is being distorted by their circumstances.

Culture, opportunity and the milestone. Milestones describe what children do when they are given the conditions to do it. A child who has not learned to count by six may be delayed, or may never have been taught; a child who does not speak in the clinic may be shy, or may speak a different language at home, or may have been told not to speak to strangers. The clinical question is always: what has this child had the opportunity to learn, from whom, in what language?

Risk and resilience. The literature's most useful finding for family therapists: the number of risk factors matters less than the balance between risk and protective factors, and the single most powerful protective factor is a relationship with at least one

consistently responsive adult. That finding is the developmental justification for family therapy, and it should be quoted in every assessment that identifies a child at risk.

1.2a The developmental history, asked well

The instrument the whole course depends on, and the one most often performed as a form. Eleven questions, each with what it is for.

One — "start at the beginning: was this a planned pregnancy, and how was it for you?" For: the parents' state at the child's beginning, which predicts a good deal about the relationship's foundation. An unplanned pregnancy in a hostile household is not a verdict; it is information.

Two — "how was the birth, and how were the first weeks?" For: birth trauma, prematurity, a mother's illness, a period of separation. The question also signals that the therapist is interested in the parents' experience, not only the child's.

Three — "who was at home in the first year, and who helped?" For: the caregiving network, which in many families is wider than the parents, and which is often invisible in the referral.

Four — "when did he first smile, sit, walk, and say a word?" For: the developmental trajectory, asked in ordinary language rather than as a checklist. Parents remember these with surprising accuracy, and their confidence in answering is itself informative.

Five — "was he different from your other children, and in what way?" For: the comparison parents make spontaneously, and the meaning they have given it.

Six — "when did you first become worried, and what made you worried then?" For: the starting point of the parents' concern, which is frequently earlier than the referral, and the event that converted a characteristic into a problem.

Seven — "what was happening in the family at that time?" For: the intersection of the child's development with the family's non-normative events. This is the question that produces the chronology's turning points.

Eight — "who has said what about it — school, clinic, family?" For: the descriptions already attached, and their authority. The answer tells the therapist what the family have been told, by whom, and how they have absorbed it.

Nine — "what have you tried, and what happened?" For: the family's own experiments, which are usually more numerous and more thoughtful than the referral suggests.

Ten — "what does he do well, and what does he love?" For: the competence description, which the referral will not contain, and which the therapy will need.

Eleven — "what would you want me to understand that I have not asked about?" For: the question the family have been waiting for, which is often where the real referral is.

How the questions are used in the session. Not read out in sequence; woven into the first forty minutes, in the family's language, with the child present where possible and asked directly at least three times. The therapist writes the chronology on a single sheet with dates, and shows it to the family at the end: "this is what I have understood; what have I got wrong?" The correction is the first collaborative act.

1.3 The clinical method: from behaviour to task

A four-step method that the rest of the course will use, and that a student can apply to any presenting behaviour.

Step one — describe the behaviour, precisely and without interpretation. Not "he is defiant" but "when his mother asks him to put his shoes on, he says no, sits down, and after ninety seconds his mother puts them on". The description should be filmable, dated, and observed in more than one setting where possible.

Step two — locate it developmentally. What is the expected behaviour at this age, in this culture, for a child with this history? The milestone grid gives the range; the developmental history gives this child's trajectory; the family's account gives the meaning. The output of this step is one of five statements: within normal variation; delayed but progressing; a regression; a symptom of distress; or impossible to tell without more information.

Step three — identify the task. What is the child trying to achieve, and what is the family being asked to allow? A nine-year-old's defiance may be an attempt at autonomy in a family where autonomy is punished; a two-year-old's tantrum is the task itself, not a failure of it; a fourteen-year-old's secrecy is the beginning of a private life. The task is not a hypothesis about cause; it is a description of what the behaviour is for, and it can be stated to the family in their own language.

Step four — test the formulation against the family's response. If the behaviour is an autonomy struggle, the family's response to a small increase in the child's choices should be observable within a week or two. If it does not change, the formulation is wrong, and the therapist revises it rather than pressing the family harder. This step is what distinguishes a developmental formulation from a developmental label.

Worked, briefly. The behaviour: a seven-year-old soiling again after two years of continence. Step two: regression, not delay, and therefore a signal. Step three: the task is not autonomy but the management of an experience the child cannot name — and the chronology shows the soiling began when the father moved out. Step four: the formulation predicts that the soiling will reduce as the child is given a way to speak about the departure, and the prediction can be tested in four sessions.

The method's use in the case assignment. The 30% developmental case analysis asks for exactly these four steps, applied to one child or adolescent, with the developmental history and a formulation. A student who follows the method will produce a defensible analysis; a student who begins with a diagnosis will produce a checklist.

1.4 Techniques: the four instruments of this course

The milestone grid, used as a reference. The grid (Figure 1.1) gives ranges by domain — gross motor, fine motor, language, social-emotional, cognitive. It is used in three ways: to locate a child's trajectory; to identify the domains in which the parents' concern is concentrated; and to notice what the parents have not mentioned, which is sometimes the more important finding. It is not used to score a child, and a therapist who reads the grid out to a parent has misunderstood its purpose.

The developmental timeline, drawn on one sheet. A horizontal line with the child's age marked in years, and above it the events the family describe: births, moves, illnesses, separations, deaths, school changes, and the points at which the presenting behaviour began or changed. The drawing is done with the family, in the session, and it is the single most useful artefact of a developmental assessment. Its power is visual: families see the coincidence of a symptom's onset with an event, which no amount of questioning produces as convincingly.

The three-setting comparison. Ask what the behaviour looks like at home, at school (or nursery, or church), and with a particular person — usually a grandmother, an aunt or a favourite teacher. The comparison answers the question the clinic cannot: is this behaviour a property of the child or of a relationship? A child who is silent at home and fluent at school is not a child with a language disorder.

The ordinary-language formulation. The therapist's developmental understanding, converted into one or two sentences a parent can use: "he is doing what six-year-olds do — testing whether the rules are the same when you are tired — and the useful response is to keep the rule and not to explain it at length." The formulation is the intervention, and its quality is judged by whether the parent can repeat it accurately a week later.

The developmental question, as a habit. "What would you expect a child of this age to be doing?" asked of the parents before the therapist offers an opinion. The question does three things: it elicits the family's developmental theory (which is often accurate), it reveals the standard against which the child is being judged, and it positions the therapist as a collaborator rather than an authority.

1.4a Two timelines, drawn and read

Timeline one — a developmental problem that is not a developmental disorder.

The case. Chidi, 6, referred by his school for "immaturity and poor concentration". The family: mother (29), father (34, works away), a new baby of four months, and a grandmother who has moved in to help.

The timeline. Birth, unremarkable. Walking at 14 months, first words at 15 months, sentences at 26 months — all within range. Nursery at three, described as "a pleasure". School at five, settled. At five years six months: the baby is born. At five years eight months: the grandmother moves in. At five years nine months: the school reports "immaturity".

The reading. Every developmental marker is within range, and there is no regression of an acquired skill. What changed was the household: a new baby, a grandmother, and a

mother whose attention is now divided three ways. The behaviour the school calls immaturity is a six-year-old's attempt to reclaim a position in a household that has reorganised without him.

The formulation, in ordinary language. "He is not behind. He is doing what a six-year-old does when a baby arrives and the house fills up: he is trying to get back the place he had. The useful response is not to expect less of him, which confirms his fear, but to give him a job with the baby that only he can do."

Timeline two — a developmental problem that is.

The case. Zainab, 4, referred by her nursery for "not talking". The family: mother (33), father (38), two older children, and a history the referral does not mention.

The timeline. First words at 13 months, two-word phrases at 20 months — unremarkable. At three years two months, the family moved cities. At three years four months, the paternal grandmother died. At three years five months, Zainab stopped using sentences. At three years eight months, she stopped speaking outside the home. At four, she speaks in single words to her mother only.

The reading. This is a regression: an acquired skill has been lost, and it was lost at a specific point. The differential is wide — hearing, neurological, trauma, selective mutism — and all of it requires assessment. The developmental formulation does not name the cause; it establishes the urgency and the sequence, and it says plainly that a child who has lost language needs to be seen by a paediatrician within weeks, not months.

The clinical action. Referral for hearing and paediatric assessment in the same week; the family work continues alongside, addressing the move and the bereavement in the child's language; the nursery is asked to stop asking her to speak and to describe what she does instead.

What the two timelines teach. The same presenting description — "not talking", "immature" — and two completely different clinical situations. The timeline is what distinguishes them, and the timeline takes fifteen minutes and one sheet of paper. A therapist who skips it will either pathologise Chidi or reassure Zainab's parents, and both errors are common. The instrument costs fifteen minutes and it is the difference between a developmental assessment and a developmental opinion.

1.4b The four categories, tested on eight descriptions

The laboratory's classification exercise, written out so that it can be revised and so that the reasoning can be examined. Classify each, and state what further information you would need.

One. A girl of 15 months has not walked; she pulls to standing, cruises the furniture, and is unconcerned. Classification: normal variation at the late end, with a review date. Information needed: birth history, any illness, family pattern of late walking, and a review in eight weeks.

Two. A boy of 26 months has twelve words; he understands instructions, points, and has begun to combine two words this month. Classification: delay, with progress. Information

needed: hearing test, whether he is being raised in more than one language, and the parents' account of who speaks to him and how.

Three. A girl of four who spoke in sentences has, over three months, stopped using sentences and now speaks in single words to her mother only. Classification: regression. Action: paediatric and audiology referral within the week; do not wait for a review.

Four. A boy of seven has begun wetting the bed after four years of being dry, and has become clingy at bedtime. Classification: symptom, with regression of a self-care skill. Information needed: what changed in the household in the last three months — illness, separation, move, a new adult, a bereavement.

Five. A girl of nine refuses to do homework, cries when her mother mentions school, and has stopped seeing her friends. Classification: symptom, and the school refusal is the presenting edge. Information needed: what is happening at school, including bullying and a specific adult; and what is happening at home that makes staying at home attractive.

Six. A boy of 13 has begun to close his bedroom door, to be private about his phone, and to prefer friends to family. Classification: normal variation — the developmental task of adolescence. Information needed: whether the privacy is accompanied by withdrawal from all relationships and by a change in functioning, which would make it a symptom instead.

Seven. A girl of 30 months has frequent tantrums lasting up to twenty minutes, several a day, in a household that has just moved and in which a new baby arrived six weeks ago. Classification: normal variation in behaviour, in a family under two non-normative events at once; the family may need support and the child does not need a referral. Information needed: the parents' own state, and whether the tantrums endanger anyone.

Eight. A boy of five is reported by his school as "unable to concentrate"; at home he plays Lego for forty minutes, and at his grandmother's house he sits through a two-hour church service. Classification: insufficient information — and the three-setting comparison already suggests that the behaviour is a property of the classroom relationship rather than of the child. Information needed: an observation in the classroom, the teacher's account, and the seating and task arrangements.

Why eight descriptions and not one. Each of the eight is decided by a different piece of information: the trajectory, the language environment, the loss of a skill, the household's recent events, the school context, the developmental task of the age, the number of stressors, and the cross-setting comparison. Students who can classify all eight, and name what they would need to know, have the course's first instrument.

1.5 The first session with a developmental concern

A structure for the assessment session, in five phases, with the words a therapist can use.

Phase one — the frame (five minutes). "I am going to ask about your child's development, and about what has been happening at home, because the two are connected. I will also want to see him, and I may want to speak to the school with your permission. At the end I will tell you what I think and what I would suggest." The frame

matters more in developmental work than in any other, because parents arrive expecting to be assessed themselves.

Phase two — the concern, in the parents' words (ten minutes). "Tell me about it in your own words, not the school's." The therapist writes the family's language and the referral's language in two columns, and notes the difference. The difference is usually informative: the school says "defiance", the mother says "he is angry with me since his father left".

Phase three — the developmental history (twenty minutes). The eleven questions of 1.2a, woven rather than read, with the child present and asked directly at least three times. The therapist draws the timeline as the answers arrive, and turns the sheet so the family can see it.

Phase four — the observation (ten minutes). Not a test: an observation of the child with a task and with the parent. A simple task — a puzzle, a drawing, tidying up — is enough to see the child's attention, his language, his response to a request and his response to his parent's response. The therapist narrates what she sees aloud, in neutral language: "I notice he looks at you before he answers." The narration is the assessment, and it is also the intervention, because it teaches the parent to look.

Phase five — the formulation and the next step (five minutes). One of the five statements from 1.3, in ordinary language, plus a next step: a paediatric referral, a school observation, a further session, or a plan. The formulation must include the family's competence: what they have already done that has helped.

What the therapist does not do in session one. Does not score a developmental test; does not name a diagnosis; does not tell the parents what they should have done; does not reassure them that everything is fine before the assessment is complete; and does not leave without a next step, and never ends a first session with a family who are unsure whether the therapist believed them.

The three sentences parents remember. "This is not your fault, and it is not nothing." "Here is what I think is happening, in one sentence." "Here is what we will do next, and when."

1.6 Case: the Okonkwos, a developmental reading

The course's recurring family, read developmentally, which is a different reading from the one any therapy model gives.

The family. Ebuka (9), his mother Ada (34), his father Chuka (39, works nights), and a baby of eleven months. The paternal grandmother died in 2023. The presenting complaint is defiance.

The chronological reading. Ebuka's developmental history is unremarkable: walking at 13 months, words at 16 months, reading at six, no illnesses, no separations. The family's non-normative events are the grandmother's death in 2023 and the baby's birth eleven months ago. The defiance began four months after the baby's birth, and intensified when Chuka changed to night shifts.

The developmental task. At nine, the task is industry: to become competent at something that matters, in the company of peers and adults who recognise the competence. The family's task is to provide a domain in which the child can be competent, and a structure in which his competence is visible. In this household, the domain has been removed: the grandmother who taught him to cook died, and the mother's attention is with the baby. The defiance is not a failure of industry; it is an attempt to be noticed in a household where the noticing has moved.

The task-stall, named. "He is trying to get back into the place he had before the baby and before your mother died, and the only lever he has found is refusing." The sentence is offered to the parents as a formulation, and it is testable: if the formulation is right, giving him a recognised role with the baby and restoring one of the grandmother's activities should reduce the defiance within three weeks.

What the reading adds to the therapy models. Week 4 of MFT 702 would describe a boundary problem; Week 6 would hypothesise a function; Week 8 would write a behavioural plan. The developmental reading supplies something none of them does: the content of the intervention. Not "spend more time with him" but "teach him to cook the one dish your mother taught him, and let him cook it on Sundays", which is the grandmother's function restored, the child's industry given a domain, and the family's grief given a place.

The three-week test. At the review: has he cooked twice? Has the defiance fallen on those days? If the answer is no to both, the formulation is wrong, and the therapist says so and revises.

1.6a The same child, three developmental misreadings

Misreading one — the diagnosis. The school's description ("possible ADHD") is accepted, and a referral for assessment is made without a developmental history. The assessment finds no disorder, three months are lost, and the family's confidence in services falls. The error: a description was treated as a finding. The correction: the timeline first, the referral second, and the referral framed as a question rather than a conclusion.

Misreading two — the reassurance. The therapist, noting that all milestones are within range, tells the parents that Ebuka is "a normal nine-year-old" and that the behaviour will pass. The parents hear that their distress is unwarranted, and they do not return. The error: the developmental assessment answered a question nobody had asked (is he delayed?) and ignored the question they had (what is happening to my family?). The correction: both statements, in this order — "his development is on track, and what is happening at home is real and worth working on".

Misreading three — the moral reading. The defiance is treated as a discipline problem, and the intervention is a firmer consequence. The behaviour worsens, the mother's confidence collapses, and the father's absences increase. The error: a developmental signal was read as a character defect. The correction: the task question — what is the behaviour for? — asked before any consequence is designed.

The pattern across the three. Each misreading is produced by skipping one step of the method in 1.3: the diagnosis skips the description and the timeline; the reassurance skips the task; the moral reading skips the developmental location. The method is short, and every step of it is load-bearing: skip one and the assessment produces an opinion instead of a formulation.

1.7 Annotated transcript: the timeline drawn

Twelve turns from the first session with the Okonkwos, while the therapist draws the timeline. The transcript shows how a developmental history is actually taken.

> **T1 Therapist:** I want to draw a line, and put on it everything that has happened since Ebuka was born. Can we do that together? When was he born?
> **T2 Ada:** March 2016. He was fine, everything was fine.
> **T3 Therapist:** (writing) Fine. When did he walk?
> **T4 Ada:** Thirteen months. He was quick.
> **T5 Therapist:** And talking?
> **T6 Ada:** Sixteen months. He talked early too. His grandmother used to say he talked for the whole family.
> **T7 Therapist:** (pause) Tell me about his grandmother.
> **T8 Ada:** She died in 2023. In August. She lived with us.
> **T9 Therapist:** (writing, then turning the sheet) So — here is 2023, and here is August. What happened in the house after that?

> **T10 Ada:** Everything changed. She used to cook with him. She used to sit with him in the evening and he would tell her everything.
> **T11 Therapist:** And when did the problems start?
> **T12 Ada:** (long pause) After the baby. But — now that I am saying it out loud — he stopped telling me things when she died.

Annotations. T1 frames the timeline as a joint activity, which converts an interrogation into a drawing. The paper is turned toward the family throughout, and T9's "turning the sheet" is the moment the instrument does its work.

T3–T6 are the milestones, asked in ordinary language, and they are answered with confidence — which is itself a finding: this mother knows her child's development, and her competence is visible to the therapist and, more importantly, to her.

T7 is the pivot, and it is earned: the therapist follows the grandmother's name rather than the schedule. The grandmother appears in the mother's answer as a person with a relationship to the child, which is the most important thing said in the session.

T9 is the timeline's use: the therapist locates the event in time and asks what happened next, which is a different question from "how did you feel". T10 produces the content of the intervention: cooking, and evening conversation.

T12 is the session's turning point, and it happens because the timeline made the sequence visible. The mother corrects her own account — "after the baby… but now that I am saying

it out loud" — and the correction is hers, not the therapist's. That is what a timeline does that a question cannot: it makes the family's own sequence visible to the family, and the correction that follows belongs to them.

What the trainee should notice. The therapist asks eleven questions and makes no interpretation. The one interpretive act in the session is the mother's, produced by the drawing. The instrument does the work, and the therapist's skill is in holding the paper and following the family's own order of significance.

1.8 Laboratory: the timeline and the four categories

Sixty minutes, in pairs and then in the whole group.

Part one — the timeline from memory (fifteen minutes). Each student draws their own developmental timeline, then interviews the partner using the eleven questions, drawing the partner's timeline. The drawing, not the notes, is the output.

Part two — classification (twenty-five minutes). The lecturer presents eight short cases, each a description of a behaviour with a minimal history. Students classify each as normal variation, delay, regression, symptom, or insufficient information, and must state what further information they would need. Two of the eight are deliberately unclassifiable.

Part three — the sentence (twenty minutes). For each of four cases, each student writes the ordinary-language formulation in two sentences and reads it aloud. The group judges whether a parent could repeat it a week later.

Marking rubric for the 30% case analysis.

Criterion	Weight	Full marks
Description	15%	Behaviour described precisely, in more than one setting, without interpretation
Developmental location	25%	Correct category, with the evidence for it and the differential acknowledged
Task formulation	20%	The person's task and the family's task stated, in testable form
History and chronology	20%	Timeline with normative and non-normative events, and the onset located
Ordinary language	10%	A formulation a parent could repeat
Limits	10%	What the formulation cannot tell you, and what would change it

The distinction-level answer. It distinguishes delay from regression with evidence; it names the task in a sentence the family could use; it locates the onset against the family's events; it states what would falsify the formulation; and it does not diagnose.

1.9 Six errors in developmental assessment

1. **The checklist.** Running the milestone grid as a questionnaire, which produces scores without a child. Fix: the grid as a reference, the timeline as the instrument.

2. **The premature diagnosis.** Naming a condition in the first session, which forecloses the assessment and organises the family around a label. Fix: the five statements of 1.3, and a referral framed as a question.
3. **The false reassurance.** "He will grow out of it", offered before the assessment is complete. Fix: two statements — development is on track, and the family's difficulty is real.
4. **The missed regression.** A lost skill attributed to stubbornness. Fix: regression is a red flag; refer within days.
5. **The decontextualised child.** Assessing the child alone, in a clinic, without the household, the school or the language. Fix: the three-setting comparison, and at least one session with the caregiving network.
6. **The moral reading.** Treating a developmental signal as a character defect. Fix: the task question, asked before any consequence is designed.

1.10 Culture, faith and development

The cultural question, stated baldly. Almost every developmental milestone in this book was established by studying children in Europe and North America, and almost every one of them is a statement about what children do when they are fed, housed, spoken to and played with in a particular way. That does not make the milestones false; it makes them conditional, and the condition is the child's opportunity.

Three examples where the condition matters.

Language. The vocabulary and grammar norms in the literature come from children learning a single language in a household where adults speak to them directly. A child in a multilingual household, or a household where adults speak to each other and not to children, may be developing normally and score below the norm. The clinical error is to call this delay; the clinical task is to assess competence in the child's own language, with a speaker of that language.

Autonomy. The two-year-old's task is described in the literature as autonomy, and in many households the task is instead obedience, and the child who asserts autonomy is corrected earlier and more firmly. The child is not delayed; the family's developmental curriculum is different. The therapist who treats the family's curriculum as the problem will lose the family, and the therapist who treats the child's autonomy as the only healthy outcome will impose a value.

Sleep and independence. The literature on infant sleep assumes a separate sleep surface and a self-settling child. In households where infants sleep with their mothers, the milestone's behavioural markers are absent and the developmental process is not. A therapist who recommends "sleep training" to such a family is not offering developmental science; they are offering a cultural practice.

What the competent practitioner does. Three things. First, asks the family what is expected of a child of this age in their household, and what happens when a child does not meet it. Second, assesses competence in the child's own language, with the family's own

tasks, before comparing to a norm. Third, names the translation when they make one: "in the studies this comes from, children are spoken to directly by adults; in your house it is different, so I am going to test something else."

Faith and development. Religious households have their own developmental curriculum: when a child prays, fasts, reads scripture, is initiated, and takes a place in the community's rituals. These are developmental tasks in the family's terms, and they matter to the child's identity as much as school competence does. The clinician's interest is not in the content of the curriculum but in its fit with the child: a child who is required to fast before puberty may be perfectly well; a child required to carry an adult's religious anxiety is not, and the difference is the child's own state, not the practice.

The examinable sentence. "The milestones are conditional on opportunity, and the clinical question is what this child has had the chance to learn, from whom, in what language." A student who can say that and apply it to a family in front of them has understood the course's cultural premise.

> **DEBATE**
> "Developmental norms are a tool of cultural standardisation, and a family therapist should use them only with reluctance." Argue both sides, and say what you would lose by abandoning them.

1.10a The course's four categories, applied to a family

A worked application of the four categories to the Adeyemi family, because the categories are usually taught with individual children and used with families.

Normal variation, in a family. A fourteen-year-old who wants to spend Saturday with friends rather than relatives is doing the developmental task of the age. The family experiences it as rejection. The clinical response is to name the task, to the parents, and to distinguish it from the conflict that may surround it.

Delay, in a family. A family in which the parents have not yet made the transition from managing a child to negotiating with an adolescent: the developmental task of the parents is delayed. The adolescent's behaviour is age-appropriate and the family's organisation is not, and the formulation must say so without blaming the parents, who may have had no model of adolescence in their own upbringing.

Regression, in a family. A family that had established a workable routine, then lost it when a member died: the household's functioning has regressed to an earlier, less differentiated arrangement. The regression is a signal, and it tells the therapist that the family's coping has been overwhelmed rather than that the family is deficient.

Symptom, in a family. A child's school refusal that functions to keep a parent at home during a marital crisis is a symptom — a signal of the system's distress — and it is neither normal variation nor delay. The clinical response is to treat the signal as information about the system, which is exactly what Week 4 of MFT 702 argues.

Why the family application matters. Students who learn the categories on individual children classify individual children and then treat families as if the categories did not apply to them. They do: households have developmental tasks, and households can be delayed, regressed, symptomatic or simply varied. The therapist who can say "your family is doing what families do at this stage, and the difficulty is that the stage has arrived early" has a formulation the parents can use.

1.11 Evidence note: what developmental science can and cannot support

What is well established. The sequence of gross motor and language milestones, with wide normal ranges; the importance of responsive caregiving in the first years; the association between early adversity and later difficulties, with the caveat that the association is probabilistic; the protective effect of at least one consistent relationship; and the finding that development is bidirectional, with child characteristics shaping parenting as much as the reverse.

What is contested. The size and reversibility of early adversity's effects (the "first thousand days" literature is stronger on the first claim than the second); the predictive value of infant temperament for adult personality; the specificity of attachment classifications for later disorder; and the extent to which screeners improve outcomes, as distinct from identifying children.

What is often misused in clinical settings. Milestone checklists presented to parents as though they were diagnostic; "brain development" arguments used to pressure parents into particular practices; and attachment language used to describe ordinary family difficulty. A family therapist should be able to recognise each of these, because each will be used on the families they see.

What the developmental literature gives a family therapist. Three things of real value: a timeline against which to locate a concern; a set of tasks that make behaviour intelligible; and the strongest evidence base in the field for the claim that relationships are the mechanism of development, which is also the claim that justifies family therapy.

The matrix cell, in this course's terms. "Strong descriptive evidence for sequences and ranges; moderate evidence for caregiving's role; contested evidence for early-adversity determinism; and no evidence for any single parenting practice as universally optimal." A student who writes that in an examination will do well.

The practitioner's claim. "I use developmental knowledge to locate a behaviour and to distinguish variation from delay from regression from symptom; I use the timeline as my main instrument; I do not diagnose from milestones; and I assess competence in the family's own language and tasks before comparing to a norm." That sentence is the course's professional baseline, and it is also the sentence to say in a case discussion when a colleague reaches for a diagnosis before a history.

1.11a What development cannot explain

A short section, because the developmental frame is powerful and can be overreached.

Development cannot explain a family's meaning. That a child is nine tells you about industry, peers and concrete-to-abstract thinking; it does not tell you what the child's defiance means in a household where his grandmother has died. The developmental frame supplies the stage; the family supplies the meaning. A therapist who explains the second by the first is committing the error this course exists to prevent in the opposite direction.

Development cannot explain a symptom's function. A regression is a signal, and what it signals may be a developmental event, a family event, or both. The developmental reading locates the timing; the systemic reading explains the function. The two are complementary and neither is sufficient.

Development cannot adjudicate values. Whether a child should sleep alone, be circumcised, be punished physically, be taught scripture, or be allowed to choose her own friends are questions about which developmental science has some evidence and no authority. The practitioner's obligation is to know the evidence, state it, and distinguish it from their own values — which is the same obligation this book's later courses will examine in ethics and culture.

The sentence to keep. "Development tells me when and what to expect; the family tells me what it means; and I must not confuse the two, in either direction."

1.12 Key terms

Term	Definition
Developmental task	The challenge a stage presents to a person, and the matching negotiation it presents to the family
Bidirectional development	Child and caregiver shape each other; neither is the sole cause
Normal variation	Behaviour within the expected range, differing in style and pace
Delay	Functioning below the expected range, but progressing
Regression	Loss of a previously acquired skill; a red flag
Symptom	Behaviour signalling distress rather than development
Normative event	An event expected at a stage (school entry, puberty, leaving home)
Non-normative event	An unexpected event (bereavement, migration, illness, war)
Developmental history	A chronology of milestones and family events, used to locate a concern in time
Timeline	The one-sheet drawing of the chronology, made with the family
Three-setting comparison	Comparing behaviour at home, elsewhere, and with a specific person
Opportunity condition	The assumption that milestones describe children who have had the chance to learn

1.13 Self-test

1. Name the three clinical errors that developmental literacy prevents, with an example of each.

2. Distinguish delay from regression, and say why the distinction changes the urgency.
3. What are the four steps of the method in 1.3?
4. Give five of the eleven developmental-history questions and what each is for.
5. Why is the three-setting comparison useful, and what does it rule out?
6. State the bidirectional claim and why it matters to a family therapist.
7. What does the milestone grid not tell you?
8. How does a therapist handle the cultural condition on the milestones?

Answers. 1. Pathologising the ordinary (a two-year-old's "no"); missing the abnormal (a five-year-old who stops speaking); mis-timing the intervention (reasoning with a three-year-old). 2. Delay is below range but progressing and permits assessment over weeks; regression is the loss of an acquired skill and requires referral within days. 3. Describe the behaviour precisely; locate it developmentally (one of five statements); identify the task for the person and the family; test the formulation against the family's response. 4. Any five: the pregnancy and its context; the birth and first weeks; who was at home and who helped; milestones in ordinary language; how the child differed from siblings; when the parents first became worried; what else was happening then; who has said what; what has been tried; what the child does well; what has not been asked. 5. It establishes whether the behaviour is a property of the child or of a relationship, and it rules out a generalised language or behavioural disorder when the child is fluent elsewhere. 6. Child and caregiver shape each other continuously; it matters because the clinical unit is the relationship, and because blaming either party misreads the evidence. 7. It cannot distinguish causes — deprivation, illness, trauma, family crisis — nor diagnose; it identifies children who need fuller assessment. 8. Ask what the family expects of a child of this age; assess competence in the child's language and tasks; name the translation when comparing to a norm.

1.14 Reflective task

Draw your own developmental timeline from memory, then ask the person who raised you to correct it. Write two paragraphs: one on what you had forgotten, and one on what your family remembered differently. The exercise is the course's first instrument applied to the student, and most people discover that their own developmental history is a family story with a particular narrator — which is exactly what the families in this course will present to them. Keep the corrected timeline: it is the first artefact of your own developmental literacy, and you will read it again in Week 15.

1.15 Readings for this week

- **Berk (2017)**, Development Through the Lifespan, the reference text for the course, read the opening chapters for the frameworks and keep it for the milestones.
- **Erikson (1963)**, Childhood and Society, for the task language, read as a historical document as well as a theory.
- **Bronfenbrenner (1979)**, The Ecology of Human Development, for the nested contexts that make the family's environment part of the child's development.

- **Sroufe (2005)**, "Attachment and development", for the evidence on continuity and change.
- **Rogoff (2003)**, The Cultural Nature of Human Development, the essential corrective to the norms, and the source of this week's cultural argument.
- **Sameroff (2009)**, The Transactional Model of Development, for the bidirectional claim in its strongest form.

Read Rogoff first and Berk second; the corrective before the reference. For the case analysis, use at least one source that disputes a norm you are relying on, and say so in the paragraph on limits. The reading-log question this week is: "which of my own developmental assumptions did this reading make visible?"

Next: Week 2 — Prenatal development, temperament and becoming a parent

---

Week 2 — Prenatal development, temperament and becoming a parent

Prenatal and perinatal. Brain development, maternal stress and nutrition, birth experience, and the "fourth trimester". Clinically relevant: prematurity, birth trauma, postnatal depression (which affects up to 1 in 5 mothers in some settings and is strongly associated with relational and child outcomes), and paternal postnatal depression.

Temperament. Nine dimensions (Thomas & Chess) usually simplified to three profiles: easy, slow-to- warm, and difficult/feisty. Temperament is not destiny but it is **goodness of fit** that matters: a highly sensitive child in a loud, unpredictable household will struggle more than the same child in a predictable one.

The transition to parenthood. The couple must reorganise: time, sleep, money, division of labour, sexuality, relationship with grandparents, and identity. Research consistently finds relationship satisfaction dips after the birth of a child — and that the quality of the co-parenting alliance, more than the amount of conflict, predicts child outcomes.

> **CASE**
> A couple presents with "no time for each other" four months after their first baby. The developmental frame normalises the strain (this is the most common point of relational collapse), then asks the systemic question: which grandparent has been recruited into the parenting triangle, and what did the couple stop negotiating when the grandmother moved in?

Practice. In pairs, conduct a 10-minute temperament interview with a parent (or peer playing one): sleep, feeding, reaction to change, intensity, mood. Then identify the goodness of fit with the household.

2.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Before birth	The prenatal environment, and why a family therapist cares	Name one prenatal factor in a family they know
10–25	Prematurity and the fourth trimester	The relational consequences of a medical beginning	Map a premature infant's caregiving network
25–40	Postnatal depression	One in five mothers; the relational mechanism; the father's version	Screen a volunteer with a two-question tool
40–55	Temperament	Nine dimensions, three profiles, and goodness of fit	Interview a peer as a parent
55–70	Becoming a parent	The reorganisation: time, money, sex, labour, grandparents, identity	Draw the reorganisation on one page
70–82	The co-parenting alliance	Why it predicts child outcomes better than conflict	Rate three recorded couple clips
82–90	The clinical frame	Normalising the strain without dismissing it	Write the normalising sentence

The teaching decision that makes the week work. This week's material is the most likely in the course to be delivered as a list of risk factors, and the most useful version of it is a demonstration of how a normal developmental event becomes a clinical presentation. The lecture should take the commonest presentation in perinatal work — "we have no time for each other" — and show that it is simultaneously ordinary, dangerous, and responsive to a small intervention. The class should leave able to say what is normal about the fourth month after a first baby, and what is not.

The sentence the lecture must land. "The birth of a child is a developmental event for the parents, not only for the child." Most of this week follows from it: the symptoms are the parents', the tasks are the parents', and the assessment must include the couple's reorganisation, not just the baby's milestones.

2.2 Core concepts

The prenatal environment. The foetus develops inside an environment made of the mother's nutrition, stress hormones, illness, substances and medical care, and that environment is itself shaped by her circumstances — poverty, work, a violent household, a wanted or unwanted pregnancy, a partner's support. The clinical relevance is not that prenatal events determine later life but that they are the first chapter of the family's story, and families bring that chapter to the clinic, often without knowing they are doing so.

The fourth trimester. The three months after birth, in which the infant is neurologically immature and utterly dependent, and the parents are in a state of physiological and emotional reorganisation. Sleep deprivation, hormonal change, the loss of an established identity and the arrival of an unremitting demand are all normal. The concept is clinically useful because it converts a set of complaints into a stage.

Prematurity and the medical beginning. A premature birth changes the sequence: the parents' first relationship with their child is often mediated by machines, by staff, by fear, and by a period of separation. The consequences are relational rather than deterministic:

parents may be more anxious, more inclined to monitor, less confident in their own competence, and the infant may be more reactive, harder to read, and slower to settle. The clinical task is to restore the parents' confidence in their own reading of their child, which is the thing the neonatal unit has, understandably, taken from them.

Birth trauma. A difficult birth can leave a mother with a sense of violation, a fear of the body, and a reluctance to speak about it — often because she has been told she should be grateful. The clinical task is to ask, specifically, and to let her tell it. The question "how was the birth?" asked with time to answer is one of the most useful questions in perinatal work.

Postnatal depression. Affecting roughly one mother in five in many settings and a substantial minority of fathers, postnatal depression is the most clinically consequential condition in this week's material. Its mechanism is relational: a depressed mother is less responsive, the infant becomes more distressed and harder to soothe, the father feels excluded or overloaded, and the couple's negotiation collapses. The screening questions are short and should be asked of both parents at every perinatal contact.

Paternal postnatal depression. Less recognised and equally real: irritability, withdrawal into work, alcohol use, and a refusal to describe the state as depression. The clinical presentation is often a couple conflict, and the diagnosis is missed because nobody asks the father.

Temperament, and the nine dimensions. Thomas and Chess described nine dimensions — activity level, regularity, approach or withdrawal, adaptability, sensory threshold, intensity of reaction, mood, distractibility and persistence — usually simplified into three profiles: easy, slow-to-warm, and difficult (or feisty). The dimensions matter clinically because they describe the child's contribution to the relationship, which many parents experience as a verdict on their parenting.

Goodness of fit. The concept that saves temperament from fatalism: the outcome depends on the match between the child's characteristics and the environment's demands. A highly sensitive child in a loud, unpredictable household struggles; the same child in a predictable household thrives. The clinical implication is that the intervention may target the environment rather than the child, which is a systemic move disguised as a developmental one.

The transition to parenthood. The couple must renegotiate seven domains: time, sleep, money, the division of labour, sexuality, the relationship with grandparents, and identity. The research finding that satisfaction dips after the birth of a child is robust across cultures, and the dip is not a sign of a failing marriage; it is a stage. What distinguishes couples who recover from those who do not is not the amount of conflict but the quality of the co-parenting alliance.

The co-parenting alliance. The couple's capacity to coordinate as parents — to support each other's relationship with the child, to present a united approach without undermining each other, and to keep the child out of their conflicts. It predicts child outcomes more strongly than marital satisfaction does, which is why perinatal work that focuses only on the couple's romance misses the mechanism.

Becoming a parent as an identity transition. The arrival of a child requires the adult to become someone new, and the new identity is built from the models available: their own parents, their culture's expectations, their partner's expectations, and their own ideals. The clinical presentations are recognisable: the parent who cannot bear to be like their own mother and over-corrects; the parent who has no model at all; the parent whose identity was entirely occupational and who now has none.

The couple's reorganisation as the unit of assessment. The perinatal assessment is not the baby's milestones; it is the household's reorganisation. Who sleeps when; who feeds; who works; who decides; who else is in the house; what has been given up; and what the couple have stopped negotiating. The last question is the most productive, and it is the one the week's case turns on.

2.2a The goodness-of-fit grid, filled in

A worked instrument: the temperament interview, the household's demands, and the fit between them, on one page.

The child. Three months old, first baby. Dimensions (parent's report): high activity; irregular sleep; withdraws from new sounds; slow to adapt to a change in routine; low sensory threshold (startles at the door); intense reactions (screams, not cries); predominantly negative mood in the evening; hard to distract; persistent.

The profile. Difficult or feisty, by the classical typology — and the therapist should say "this describes a pattern, not a verdict on your baby" when reporting it.

The household's demands. People: mother, father, and the father's mother, who has moved in "to help". Rhythm: the father works a rotating shift, so the household's week changes every seven days. Noise: a shared compound with four other households and a generator. Space: one room for the family. Support: the grandmother is present but her methods differ from the mother's, and the mother does not feel able to say so. Sleep: the mother is doing all night feeds because the father "needs to be fresh for work".

The fit, in four statements. First, this infant's low threshold is being met by a high-noise environment, which produces more crying, which produces more maternal exhaustion. Second, the infant's irregularity is being met by a household whose week changes every seven days, and no routine survives. Third, the infant's intensity is being read by the mother as rejection ("he hates me by six o'clock"), which is the meaning that matters more than the temperament. Fourth, the evening negativity coincides with the grandmother's arrival home, which the mother has noticed and not said.

The intervention, derived from the fit rather than from the child. Not a sleep programme, not a referral. Four changes: a predictable evening hour with the light low and the door closed, negotiated with the grandmother; the father taking the six-to-nine period on his rest days, which is a co-parenting act rather than a favour; one sentence the mother can say to the grandmother, rehearsed in the session; and a re-description of the evening ("he is not rejecting you; by six o'clock he has had a day and he needs less, not more").

What the grid demonstrates. That the same infant in a different household would present differently, and that the intervention targets the fit. A therapist who treated the temperament as the problem would have produced a programme the household could not run; a therapist who treated the household as the problem would have blamed a family that is doing a great deal well.

2.2b The nine dimensions, with the question that asks each

The dimensions are usually taught as a list. They are more useful as nine questions a therapist can ask a parent in fifteen minutes.

Dimension	The question	Why it matters clinically
Activity	"Is he usually on the move, or does he stay still and watch?"	A high-activity child in a small flat with a tired mother is a fit problem, not a behaviour problem
Regularity	"Does he sleep and feed at the same times each day?"	Irregularity plus a shift-working household produces a fortnightly crisis
Approach/withdrawal	"What does he do with a new food, a new person, a new room?"	Withdrawal is read by parents as rejection or shyness; the useful response differs
Adaptability	"How long does it take him to settle after a change?"	Slow adaptation plus frequent change is a formula for distress
Sensory threshold	"Does he startle at the door, the generator, a raised voice?"	Low threshold is the dimension most often mistaken for "spoiling"
Intensity	"When he is upset, is it a whimper or a scream?"	Intensity determines the household's exhaustion and the parent's sense of competence
Mood	"Is he mostly cheerful, or mostly serious and cross?"	Negative mood in the evening is normal and is read as rejection
Distractibility	"Can you settle him by showing him something else?"	The one dimension that is a resource: a distractible child is easier to soothe
Persistence	"If he wants something, does he give up or keep going?"	Persistence is a strength in a task and a problem in a battle of wills

How to use the table. Not as an assessment instrument with scores, but as a repertoire, so that the parent leaves with a description of their child that is specific, neutral and recognisably true. A parent who says "he is difficult" can be given nine sentences that are more accurate and less judgemental, and the nine sentences are the beginning of the intervention.

The two dimensions that produce the most relief. Sensory threshold and intensity. They are the two the household experiences most acutely, and the two most often moralised ("he is spoiled", "he is a screamer"). Naming them as characteristics rather than choices changes the parent's relationship to the behaviour within one conversation.

2.3 Postnatal depression: the assessment

The condition is common, consequential and under-detected. A structure for asking about it.

Who to screen. Every mother at every perinatal contact, and every father at least once, separately, with the baby out of the room and the partner absent. The separate conversation is the instrument; a joint conversation will not produce an honest answer from either parent.

The screening questions. Two, used as a first filter in a family therapy session (a validated tool is used where the service requires one): "In the past two weeks, have you often been bothered by feeling down, depressed or hopeless?" and "Have you often been bothered by having little interest or pleasure in doing things?" A yes to either is followed by "how much is this affecting your day-to-day life?" and, where the answer is substantial, by a validated measure and a referral.

The three questions that reveal the relational picture. "Who knows how you are feeling?" "What happens in the house when you have a bad day?" "What does your partner do when you cry?" The answers describe the support available, the household's capacity to accommodate distress, and the couple's pattern — which is where the family therapy begins.

The paternal screen. "How are you sleeping?" "What do you do when you feel low?" "Who would you tell?" Fathers rarely use the word depression; the observable signs are irritability, working longer hours, drinking more, and a withdrawal from the baby's care.

The risk questions. Thoughts of harm to self or the baby are asked directly, of both parents, and the answer is recorded and acted on. A therapist who does not ask because asking feels intrusive has misunderstood the duty.

The differential. Not every exhausted mother is depressed. Sleep deprivation, thyroid disease, anaemia, grief, an unsupportive partner, a violent household and a baby's illness all produce the same presentation, and the family therapist's task is to screen, to refer where the picture is clinical, and to work on the relational and practical conditions in parallel.

The boundaries of the family therapist's role. Treating postnatal depression is a clinical task that requires a clinical service; the family therapist's contribution is the couple's alliance, the household's reorganisation, the father's inclusion, and the practical support. Saying that clearly at the start prevents the two most common failures: a family therapist attempting to treat a depression alone, and a clinical service treating a depression in a household that is falling apart.

2.3a The couple's reorganisation, drawn on one page

The instrument the week's case turns on, and the one a couple can complete in fifteen minutes.

The seven domains, with the question for each. Time: what do you do together now that you did before? Sleep: who gets up, how often, and what happens the next day? Money: who earns, who decides, and what has changed? Labour: list everything that has to happen in a week and who does it. Sexuality: what has happened to it, and who has said what about it? Grandparents: who is in the house or in the decisions, and how was that decided? Identity: who are you now that you were not before, and what have you lost?

The drawing. A circle with the seven domains around it, and in the middle the couple's answer to one question: "what have you stopped negotiating?" The answers are usually specific and painful: "who gets a night off"; "whether your mother stays"; "whether I go back to work"; "whether we have another".

Why the drawing works. It converts a diffuse complaint ("we have no time") into seven specific negotiations, of which usually two are live and five are being avoided. The live ones are the therapy's agenda, and the avoided ones are the reason the live ones cannot be settled.

The grandmother question, which is the week's case. When a grandparent moves in to help, the couple's negotiation is displaced: the decisions that were theirs are now made in a triangle, and each partner's relationship with the grandparent becomes a route to influence. The clinical questions are: who invited her; who can ask her to leave; what does each partner gain from her presence; and what did the couple stop deciding when she arrived? The last question is usually answered with a silence, and the silence is the finding, and the therapist's next sentence should be the one that makes it speakable: "I am not asking you to blame anyone. I am asking what the two of you stopped deciding."

The intervention, in one sentence. "Before we talk about time for each other, I want to know what the two of you stopped deciding when your mother moved in, because that is where the time went."

2.4 The first session with a couple four months after a birth

The commonest perinatal presentation, worked through, with the developmental frame used first and the systemic question second.

The presentation. Aisha (31) and Sam (34), married four years, one baby of four months. Aisha's complaint: "we have no time for each other and he does not help." Sam's complaint: "she is never happy with anything I do." Both are exhausted. The baby is present and is being passed between them.

Phase one — the normalisation (ten minutes), done carefully. "What you are describing is the commonest thing I see in the first year, and I want to say why before we go further. At four months, neither of you has slept properly since the birth, you are both doing a job you have never done before, and the negotiations you used to have — who does what, when — are being made at two in the morning with a baby crying. That is not a failing marriage. It is the stage."

Why this is not a dismissal. The normalisation is followed immediately by the second sentence: "and it is also the point at which couples get into patterns that last for years, so it is worth working on now."

Phase two — the seven domains (twenty minutes). The reorganisation drawn on one page. Time: none together. Sleep: Aisha does every night feed. Money: Sam has taken a second job, so he is home less. Labour: Aisha does everything in the house and Sam does the shopping. Sexuality: untouched since the birth, and neither has raised it. Grandparents: Sam's mother stays four nights a week "to help". Identity: Aisha has

stopped working and says she "does not know who she is"; Sam says he is "the one who is never there".

Phase three — the question (ten minutes). "What did the two of you stop deciding when your mother started staying four nights a week?" Silence. Then Aisha: "everything. She decides the bath, she decides the feeding, and I cannot say anything because she is helping us." Sam: "I did not know you minded."

The finding. The couple's negotiation has been displaced into a triangle with the grandmother, and neither has said so. The "no time for each other" is a symptom of a negotiation that has stopped.

Phase four — a small, systemic task (ten minutes). Not a date night, which the couple cannot arrange and which would fail. Two tasks: the couple decide, together, one thing that is theirs rather than the grandmother's — the baby's bath, three nights a week — and one of them tells her, in a sentence rehearsed in the room. And Sam takes one night feed at the weekend.

Phase five — the frame (five minutes). Four sessions, with a review at the fourth; the paternal screen completed separately; a question about Aisha's mood and a referral route if the screen is positive; and the sentence to take away: "the problem is not that you have no time; it is that you have stopped deciding, and that is fixable."

What the therapist did not do. Did not take a full developmental history of the baby, which was not the presentation; did not adjudicate whether the grandmother should stay; did not suggest a date night; and did not treat the couple's complaint as trivial because it is common.

2.4a The first year, month by month: what a family therapist watches for

A timeline of the relational tasks of the first year, with the presentations that bring families to a clinic.

Months 0–1: establishment. The task is the establishment of feeding, sleep and a caregiving rhythm, in a household that has just changed shape. Presentations: feeding difficulty; a mother who cannot settle; a father who feels excluded; an uninvited relative who has taken over. The family therapist's move: the household's arrangement, drawn, and the question of who decided.

Months 1–3: the fourth trimester. The task is the parents' survival and the infant's regulation. Presentations: exhaustion presented as a relationship problem; the first arguments about nights; a mother's low mood that nobody has screened. The family therapist's move: the screen, the seven domains, and the normalisation that does not dismiss.

Months 3–6: the first negotiation. The task is the couple's re-negotiation of time, sleep and labour, and the infant's entry into social exchange — smiling, vocalising, the first games. Presentations: "we have no time"; the grandmother's methods; the father's withdrawal into work; the mother's return to work or refusal to return. The family therapist's move: the reorganisation drawing and the stopped-negotiation question.

Months 6–9: separation and mobility. The task is the beginning of separateness — weaning, crawling, the first "no", the first night away — and the parents' tolerance of the child's independence. Presentations: anxiety at separation presented as the child's problem; a parent who cannot leave; feeding battles; sleep battles. The family therapist's move: the graded separation plan, and the question of whose anxiety is in the room.

Months 9–12: the first year's reckoning. The task is the consolidation of attachment and the couple's assessment of their own first year. Presentations: a couple who have not had a conversation since the birth; a second pregnancy already underway; a decision about work that is being made in a state of exhaustion. The family therapist's move: the review, the seven domains again, and the preparation for the second year's tasks.

What the timeline is for. It gives the therapist a developmental hypothesis for any presentation in the first year, and it protects against the commonest error in perinatal work: treating a stage-specific difficulty as a relationship defect. A couple arguing about nights at month two are doing what couples do at month two, and the therapist's task is to help them do it once, well, rather than to diagnose the marriage.

2.5 Case: the prematurity that reorganised a family

A different perinatal presentation, in which the medical beginning has relational consequences.

The family. The Baloguns. A baby born at 29 weeks, now eight months old (five months corrected). The mother, Halima (27), gave up her job during the neonatal stay. The father, Yusuf (31), works away and was present for two weeks after the birth. The referral is from the paediatric clinic: "mother anxious, over-attends, does not leave the baby with anyone."

The developmental picture. The baby is thriving. She is small for her corrected age, feeding well, and meeting milestones on the corrected schedule. The paediatrician has no concern about the baby.

The relational picture. Halima has not been apart from the baby for more than forty minutes in eight months. She checks the breathing three or four times a night. She has not returned to work and says she cannot. Yusuf says she has "changed" and that the house is "a hospital now". Halima's mother comes daily and is the only person Halima allows to hold the baby.

The developmental formulation. This is not a maternal disorder; it is the predictable consequence of a medical beginning. Halima's relationship with her daughter began in an incubator, mediated by machines and staff, with a nurse telling her when she could touch her. Her monitoring is the behaviour of a mother who learned that her baby could die while she was not looking, and whose competence was, for eleven weeks, subordinate to a monitor.

The task, named for the parents. "Your job in the first months was to keep her alive, and you did it. Your job now is a different one: to let her become a person who can be looked after by other people, and that means practising being apart, in small amounts, with people you trust." The naming is the intervention's foundation, because it credits the monitoring before it asks for change.

The plan. A graded separation: forty minutes with the grandmother, twice a week, for two weeks; then two hours; then an afternoon. A breathing monitor used deliberately for two weeks and then removed, with the paediatrician's agreement, because the monitor is the mechanism of the monitoring. Yusuf given one responsibility that is his alone — the bath on the days he is home — so that he enters the care rather than commentating on it. And a conversation with Halima about her own loss: her job, her confidence, and the eleven weeks she spent watching a machine.

The outcome at four months. Halima returned to part-time work. The monitoring reduced. The couple's argument changed from "you have changed" to "we are both still frightened", which is a more workable disagreement.

What the case teaches. That a medical beginning is a developmental event for the parents, that the parents' behaviour is often the residue of competent adaptation to an emergency, and that the intervention's first move is to credit the adaptation before asking for its revision.

2.5a The couple who came at the right time, and the couple who came at the wrong time

Two brief cases, because the timing of a referral in the perinatal period determines the work.

The right time. A couple, five months after a first baby, both sleeping badly, the arguments frequent but contained, and no violence. The referral comes from the health visitor, who has noticed that the mother's mood is low and the father has stopped attending. The work is exactly this week's material: the screen, the seven domains, the stopped negotiation, and six sessions of reorganisation. The prognosis is good because the couple still have a shared project — the baby — and the conflict has not yet become the relationship's subject.

The wrong time. A couple, three weeks after a birth by emergency caesarean, referred by a relative for "not getting on". The mother is in pain and not sleeping; the baby is feeding hourly; the father has taken two days off work and is back on shift; and the referral is a relative's worry, not the couple's. Here the therapy is premature: the couple do not have a request, they have a crisis, and a session that asks them to renegotiate their relationship in week three of a newborn's life will produce compliance and no change.

What the family therapist does at the wrong time. Three things, all short. Names the stage: "you are in the hardest month, and nothing you are feeling is a verdict on your marriage." Screens both parents. And offers a single practical action that reduces the load — one night feed taken by the father, one visit postponed, one relative asked to come on a specific day rather than every day. Then a review in six weeks, when the couple can make a request of their own.

The principle. In perinatal work, the first question is not "what is the problem?" but "is this the point at which the family can work on the problem?" A family in the first six weeks of a first baby is usually in the second category, and the clinician who cannot wait will treat a stage as a pathology.

2.6 Annotated transcript: the temperament interview

Twelve turns from the temperament interview with Aisha, conducted in the first session of 2.4. The transcript shows how a developmental instrument becomes a therapeutic conversation.

> **T1 Therapist:** I want to ask about the baby's style, not about what he does wrong. Is he usually busy or still?
> **T2 Aisha:** Busy. From the moment he wakes up.
> **T3 Therapist:** And is his day predictable — does he sleep at the same times?
> **T4 Aisha:** Never. My mother says I should put him on a schedule. I have tried.
> **T5 Therapist:** What happens when you try?
> **T6 Aisha:** He screams. And then she says I am not consistent.
> **T7 Therapist:** (pause) So the schedule is not working, and the person who suggests it is in the house.
> **T8 Aisha:** (cries) Yes.
> **T9 Therapist:** Let me say something about him first. Some babies are regular and some are not, and your son is at the irregular end. That is not a fault and it is not your fault. What it means is that a fixed schedule will cost you more than it costs a mother with a regular baby.

> **T10 Aisha:** So it is not me.
> **T11 Therapist:** It is not you. And there is a second thing: how does he react when there is noise — the generator, the door?
> **T12 Aisha:** He jumps. He cries at the door. My mother says he is spoiled.

Annotations. T1 reframes the interview as being about style rather than fault, which is the single most important move in a temperament conversation. Parents arrive expecting to be assessed; the therapist asks them to describe.

T2–T4 are the dimensions, in ordinary language: activity, regularity. T4 introduces the grandmother without the therapist asking, which is the systemic material appearing inside a developmental instrument — a common and useful occurrence.

T5–T8 is the therapist following the grandmother rather than the schedule, and T7's summary — "the schedule is not working, and the person who suggests it is in the house" — is the pivot. It is neutral about the grandmother and precise about the situation, and it produces the tears.

T9 is the psychoeducation, delivered after the family's account rather than before, and it is specific: not "all babies are different" but "a fixed schedule will cost you more than it costs a mother with a regular baby". The specificity is what makes it usable.

T10 is the sentence the whole technique is for, and the therapist's answer at T11 is deliberately short and then moves on, because the point has been made and dwelling on it would make the therapist the source of the mother's relief.

T11–T12 adds the sensory dimension and produces the grandmother's second verdict ("he is spoiled"), which is the material for the next stage: a conversation about what the grandmother believes, and what the mother is allowed to say.

What the trainee should notice. The instrument produced three findings in twelve turns: an irregular, sensitive baby; a grandmother whose methods contradict the mother's; and a mother who has been holding a verdict about herself. The thermometer produced the diagnosis; the tape measure produced the household.

2.6a The prematurity case, continued: what the parents needed that the clinic did not offer

A short section on the gap between the paediatric follow-up and the family's experience.

What the clinic provided, and well. Growth monitoring, developmental review at corrected age, hearing and vision checks, and a named nurse. The paediatric follow-up was, by any standard, good.

What the clinic did not provide. Any conversation about what the eleven weeks in the neonatal unit had done to the parents. Halima had never told anyone what it was like; Yusuf had been away for most of it; and both carried a version of the experience that the other did not know. The referral had described the mother as anxious, which is a description of her behaviour and not of her experience.

The question that opened it. "Tell me about the day she was born." Not "how are you coping" — which invites a report — but "tell me about the day", which invites a story. Halima described being wheeled away from her baby, being told to rest, and being brought a photograph. She had kept the photograph in her bag for eight months. She had never told her husband what the photograph had meant.

What followed. Yusuf said, for the first time, that he had been told to go back to work and had gone, and that he had not known how to say that he wanted to stay. The couple's argument about "you have changed" became, over three sessions, an argument about a separation neither had chosen and neither had discussed.

What the family therapist contributed that the paediatric clinic could not. The clinic had the baby's history; the therapist had the parents'. The two histories were of the same event, and the second had never been taken.

The lesson for the course. A medical beginning is a family event, and the family's version of it is part of the developmental history. The practitioner who asks only about the baby's milestones will not hear it, and the parents will continue to organise their relationship around a story that has never been spoken.

2.7 Laboratory: the temperament interview and the reorganisation

Sixty minutes, in pairs.

Part one — the temperament interview (twenty-five minutes). Ten minutes interviewing a peer as a parent, using the nine dimensions in ordinary language. The

observer records which dimensions were covered, whether the therapist named a profile, and whether the naming was presented as a description or a verdict.

Part two — the reorganisation (twenty-five minutes). The same pair completes the seven-domain drawing for a family they know or have observed. The observer checks that all seven were covered and that the final question ("what have you stopped negotiating?") was asked.

Part three — the sentence (ten minutes). Each student writes the normalising sentence and the following sentence, for the couple in 2.4. The group judges whether the pair of sentences both normalises and takes the problem seriously.

Marking rubric.

Criterion	Weight	Full marks
Temperament interview	25%	All nine dimensions covered in ordinary language; profile named as description
Goodness of fit	20%	Fit named in terms of the household's demands, not the child's deficits
Reorganisation	25%	Seven domains drawn; the stopped negotiation identified
Normalisation	15%	Normalises the stage without dismissing the risk
Paternal screen	15%	Father screened separately, with the three questions, and recorded

The debrief question. "Which domain did you skip, and why?" Trainees skip sexuality and money, in that order, and the discussion of why is the week's most useful ten minutes.

2.7a The screening conversation, rehearsed

A short script for the separate conversation with each parent, which is the instrument the week depends on.

The frame. "I always speak to each parent on their own for a few minutes, because people say different things when their partner is not in the room. Nothing you tell me will be repeated without your agreement, except if I am worried about anyone's safety."

The questions, in order. "How are you sleeping?" "How are you eating?" "How is your mood — not the baby's, yours?" "In the past two weeks, have you often felt down, depressed or hopeless?" "Have you often had little interest or pleasure in things?" "Who knows how you are feeling?" "What do you do when it gets bad?" "Is there anything you have thought about that frightened you?"

The follow-up that matters. Whatever the answer, the response is the same shape: take it seriously, name it, and say what happens next. "That sounds like more than tiredness, and it is common — about one mother in five. I would like you to speak to the clinic this week, and I will keep working with you both on the household side. Would you like me to write the referral now?"

The father's version. The same questions, with two additions: "how much are you working at the moment, compared with before the baby?" and "what do you do when you cannot say how you feel?" The answers — longer hours, drinking, silence, anger — are the

paternal presentation, and naming it as a presentation rather than a character flaw is often the first relief the father has had.

What the family therapist does with a positive screen. Refers, keeps the case, and works the relational and practical side: the couple's alliance, the household's reorganisation, the father's inclusion, and the practical support. The two roles are not in competition, and saying so to the family prevents the most common confusion in perinatal work: that the therapy is either about the depression or about the marriage, when it is about both.

2.8 Six errors in perinatal work

1. **The dismissal by normalisation.** "Every couple feels like this" offered as the whole response, which leaves the family with nothing to do and their distress unheard. Fix: normalise and then take the problem seriously, in two sentences, the second of which is specific to this family rather than to families in general.
2. **The missed depression.** A mother's exhaustion attributed to the baby, and the paternal presentation missed entirely. Fix: the two screening questions, of both parents, separately.
3. **The practical prescription.** Date nights, chore charts and sleep programmes offered before the household's negotiation has been understood. Fix: the seven domains first, the prescription from the couple's own answers.
4. **The grandmother as a problem.** The helper identified as an obstacle, which puts the therapist on one side of a triangle. Fix: ask who invited her, what each partner gains, and what the couple stopped deciding.
5. **The decontextualised baby.** An assessment of milestones in a clinic, for a family in a crisis. Fix: the household as the unit, and the milestones read against the household's demands.
6. **The therapist's own developmental curriculum.** Advice that reproduces the therapist's own parenting practices — co-sleeping, sleep training, feeding — offered as developmental science. Fix: name the evidence, name its limits, and let the family decide.

2.9 Culture, faith and the perinatal period

Where the week's material travels well. The central claims — that the birth is a developmental event for parents, that the couple must renegotiate, that the co-parenting alliance predicts outcomes, that goodness of fit matters more than temperament — are supported across cultures and are useful everywhere. The screening for perinatal depression is a universal good, provided the questions are asked in a form the family can answer.

Where it strains, four ways.

The private couple. The reorganisation model assumes a household in which the couple negotiate privately. Where a grandmother, an aunt or an elder governs the household's arrangements, the couple's negotiation is not private and may not be theirs. The seven

domains are still the right instrument, and the answer to "what have you stopped negotiating?" is frequently "everything, when she arrived".

The fourth trimester as a period of rest. Some cultures prescribe a lying-in period of forty days, with the mother relieved of work and fed specially; others expect her back at the stall within a week. The practitioner should know which, because the clinical advice about rest, visitors and workload depends on it, and because a mother who returns to work on day ten against her own wishes is presenting a different problem from one who cannot leave the house.

The meaning of a difficult birth. In some families a difficult birth carries a moral or spiritual meaning — a punishment, a test, a sign. The practitioner neither endorses nor contests the meaning, and asks what the family have concluded and what they have done about it, which is the same discipline as Week 1's.

The feeding question. Infant feeding is the most reliably contentious topic in perinatal work, and the evidence is more nuanced than the argument. The practitioner's obligation is to state the evidence plainly, in both directions, to support the mother's decision, and not to conduct the argument in the session.

Faith and the perinatal period. Prayer, naming ceremonies, dedication, circumcision and blessing are developmental events in the family's terms, and they mark the child's entry into the community. The practitioner should ask about them: what will happen, when, who decides, and what it means. The question is not decorative; it frequently reveals the household's real authority structure, which is the week's central clinical question.

The examinable sentence. "The birth is a developmental event for the parents, and the household's reorganisation is the unit of assessment; culture determines what the reorganisation requires, and the practitioner's task is to ask rather than to prescribe."

> **DEBATE**
> "Perinatal services should screen every father, separately, as a matter of routine." Argue both sides, and say what the service gains and loses.

2.10 Evidence note: what is well established and what is not

Well established. The association between perinatal depression and adverse relational and child outcomes; the effectiveness of screening and of treatment; the association between the co-parenting alliance and child outcomes; the normative dip in relationship satisfaction after a birth; the heritability and stability of temperament dimensions, with stability increasing with age; and the finding that goodness of fit predicts outcome better than temperament alone.

Moderately established. The relational mechanism of perinatal depression — that the depression affects the infant through the interaction, not only through genetics; the effectiveness of couple-based interventions in the perinatal period for relationship

satisfaction; and the association between preterm birth and parenting behaviours such as over-protectiveness, with the caveat that most preterm children do well.

Contested. The size of the effect of maternal stress in pregnancy on later child outcomes; the clinical utility of the three temperament profiles as against the dimensions; whether temperament-based parenting advice improves outcomes; and the direction of the association between marital satisfaction and child behaviour, which is bidirectional and hard to disentangle.

What the practitioner may claim. "Perinatal depression is common, consequential and treatable, and screening is part of my duty; the couple's alliance matters for the child; and the household's reorganisation, not the baby's milestones, is where I work." Each of those is defensible, and none requires an overstatement.

2.11 Key terms

Term	Definition
Fourth trimester	The first three months, when the infant is neurologically immature and the parents are reorganising
Prematurity	Birth before 37 weeks; a medical beginning with relational consequences
Corrected age	Age adjusted for prematurity, used for developmental expectations
Postnatal depression	Depression in the perinatal period, affecting about one mother in five and a substantial minority of fathers
Paternal postnatal depression	The father's version: irritability, withdrawal, alcohol, refusal of the label
Temperament	Constitutionally based behavioural style, described on nine dimensions
Goodness of fit	The match between the child's characteristics and the environment's demands
Co-parenting alliance	The couple's capacity to coordinate as parents and keep the child out of their conflicts
Reorganisation	The seven-domain renegotiation required by a first child
Displaced negotiation	Decisions that have moved from the couple into a triangle with a grandparent or elder
Lying-in period	A culturally prescribed period of rest and seclusion after birth
Corrected milestone	A milestone judged against corrected rather than chronological age

2.12 Self-test

1. What is the fourth trimester, and what makes it clinically useful as a concept?
2. Name four relational consequences of a premature birth.
3. Give the two screening questions for depression, and the three questions that reveal the relational picture.
4. What is goodness of fit, and how does it change the intervention?
5. Name the seven domains a couple must renegotiate, and the question that identifies the stopped negotiation.

6. Why does the co-parenting alliance predict child outcomes better than marital satisfaction?
7. How does the practitioner handle a grandmother whose methods contradict the mother's?
8. What does the evidence support, and what remains contested?

Answers. 1. The first three months, when the infant is neurologically immature and totally dependent while the parents undergo physiological and emotional reorganisation; it converts a set of complaints into a stage with a name. 2. Any four: anxiety and hypervigilance; reduced confidence in reading the infant; a first relationship mediated by machines and staff; delayed physical contact; grief for the expected birth; a longer period before the parents feel entitled to their own judgement. 3. "In the past two weeks, have you often been bothered by feeling down, depressed or hopeless?" and "…by having little interest or pleasure in doing things?" — then "who knows how you are feeling?", "what happens in the house when you have a bad day?", "what does your partner do when you cry?" 4. The match between the child's characteristics and the environment's demands; it moves the intervention from the child's temperament to the household's rhythm, which is a systemic move. 5. Time, sleep, money, division of labour, sexuality, grandparents, identity; the question is "what have you stopped negotiating?" 6. Because the child's experience is mediated by the parents' coordination — being able to rely on each parent without being recruited into their conflict — which is a different variable from the couple's satisfaction. 7. Ask who invited her, what each partner gains, what she decides, and what the couple stopped deciding; then rehearse one sentence the mother can say, and never put the therapist in the triangle. 8. Support for screening and treatment of perinatal depression, the co-parenting alliance, the normative dip in satisfaction, and temperament's heritability; contested are the size of prenatal stress effects, the utility of the three profiles, and the causal direction between marital and child variables.

2.13 Reflective task

Ask the person who raised you one question: "what was I like as a baby?" Write down the answer verbatim, and then write three sentences on what you notice about the answer — what is described, what is remembered, and who the answer is really about. Bring it to the seminar. The exercise connects this week's temperament material to your own developmental history, and it usually produces the insight that a parent's description of a baby is also a description of a relationship.

2.14 Readings for this week

- **Thomas & Chess (1977)**, Temperament and Development, the original typology, read for the dimensions rather than the profiles.
- **Brazelton & Cramer (1990)**, The Earliest Relationship, for the clinical account of the fourth trimester.
- **Stern (1995)**, The Motherhood Constellation, for the identity transition in its richest form.

- **Cowan & Cowan (2000)**, When Partners Become Parents, the longitudinal study of the reorganisation, and the source for many of this week's claims.
- **Feinberg (2003)**, on co-parenting, for the mechanism that predicts child outcomes.
- **O'Hara & Wisner (2014)**, on perinatal mental health, for the clinical picture and the screening evidence.

Read Cowan and Cowan first and the clinical texts second; the longitudinal study will make the normalisation credible, which is the hardest thing to achieve in the room. For the case analysis, use one source that complicates the temperament advice, and say so when you discuss goodness of fit. The reading-log question this week is: "what did becoming (or not becoming) a parent teach me that no book could?"

Next: Week 3 — Infancy: attachment, synchrony and regulation

Week 3 — Infancy: attachment, synchrony and regulation

The idea. Attachment is not "bonding" and not "love"; it is a behavioural system that organises a child's strategies for seeking safety from a caregiver under stress.

The classic patterns (Ainsworth's Strange Situation; see Figure 17 in MFT 710): secure, insecure- avoidant, insecure-resistant/ambivalent, and disorganised (added by Main & Solomon, associated with frightening or frightened caregiving). In adulthood the same dimensions appear as attachment styles.

Synchrony and rupture–repair. Healthy development does not require perfect attunement — it requires frequent enough attunement and effective repair. Tronick's still-face experiments show how quickly infants respond to a caregiver's unresponsiveness, and how quickly they recover when the caregiver returns.

Co-regulation is the precursor of self-regulation. A baby does not learn to calm alone; they learn to calm with someone, repeatedly, until the pattern is internalised. This has direct clinical implications: telling an overwhelmed parent to "let him cry it out" without attention to the family's capacity and beliefs is a technique imposed on a relationship.

> **PRACTICAL**
> Watch a consented 3-minute video of a caregiver and infant (or a still-face clip). Note: gaze, vocalisation, touch, pace. Then describe one moment of rupture and one of repair in behavioural language. Avoid interpretation for the first five minutes.

3.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	What attachment is not	Three misuses of the word, from practice and from the popular press	Correct three sentences

Minutes	Segment	What the lecturer does	What students do
10–25	The behavioural system	The Strange Situation, described as an experiment	Predict four classifications from four descriptions
25–40	The four patterns	Each pattern's strategy, in the infant's terms	Write the strategy for one pattern
40–55	Synchrony and the still face	The experiment, watched, then explained	Note two ruptures and two repairs
55–70	Co-regulation	How calming is borrowed before it is owned	Design a co-regulation sequence for one family
70–82	Disorganisation	Frightening caregiving, and why it is a red flag rather than a category	Identify the clinical action
82–90	The clinical translation	What a family therapist does with this in session one	Write the sentence you would use

The teaching decision that makes the week work. Attachment is the most popularised concept in this course and the most frequently misused. The lecture should therefore begin by dismantling the popular version — that attachment is love, that it is fixed in the first year, that a child is "securely attached" as a compliment or "insecurely attached" as a diagnosis — and then rebuild it as a behavioural system with a strategy. The class should leave able to say what an attachment classification is a classification of, which is a strategy under stress, not a relationship's worth.

The sentence the lecture must land. "Attachment is a strategy, not a verdict." Everything else follows: the patterns are adaptations to what a caregiver can offer; they are changeable; and the clinical task is to understand the strategy rather than to grade the relationship.

3.2 Core concepts

The behavioural system. Bowlby's term for the set of behaviours — crying, following, clinging, checking — that a child organises in order to maintain proximity to a caregiver under threat. The system is activated by danger, fatigue, illness and unfamiliarity, and it is deactivated by the caregiver's availability. It is not a feeling and not a personality trait; it is an organised set of moves that a child makes when frightened, and the organisation is what the classification describes.

The internal working model. Bowlby's second concept: from repeated experiences, the child builds a working model of what happens when I am frightened, who comes, whether I am worth coming for, and what I must do to maximise the chance of being helped. The model is not conscious and not verbal; it is a set of expectations that shapes behaviour, and it is what transfers into adult relationships.

The Strange Situation. Ainsworth's twenty-minute laboratory procedure: the infant and caregiver are in a room with toys; a stranger enters; the caregiver leaves; the caregiver returns; the caregiver leaves again; the infant is alone; the stranger returns; the caregiver returns. The classification is made on the infant's behaviour at the two reunions, not on

the separation distress — which is the detail most often missed. What matters is how the child uses the caregiver on return.

Secure (B). The infant is distressed by separation, greets the caregiver on return, is comforted, and returns to play. The strategy: my signals work, so I use them, and I can go back to exploring. The caregiver's contribution: responsive, available, and effective when the child signals.

Insecure-avoidant (A). The infant shows little distress at separation and avoids or ignores the caregiver on return, while physiological measures often show arousal. The strategy: my signals do not bring comfort, often bring rejection, so I will not signal and will manage alone. The caregiver's contribution: frequently rejecting of the child's attachment behaviour, particularly when the child is distressed.

Insecure-resistant or ambivalent (C). The infant is highly distressed, and on return is not easily comforted; may seek contact and then push away. The strategy: my signals bring inconsistent responses, so I must amplify them and cannot settle, because settling would end the attempt. The caregiver's contribution: inconsistent availability — sometimes responsive, sometimes not, in a way the infant cannot predict.

Disorganised (D). Main and Solomon's addition: the infant shows contradictory, incomplete or undirected behaviours at reunion — freezing, stilling, approaching and then turning away, a hand to the mouth. The strategy: there is no strategy, because the source of comfort is also the source of fear. The caregiver's contribution: frightening or frightened caregiving — the parent's own fear, trauma or violence enters the interaction. This is a red flag rather than a category, and it demands assessment.

The distribution, and what it means. In low-risk community samples, roughly sixty per cent secure, with the remainder divided among the insecure categories and a small proportion disorganised. In clinical and high-risk samples the proportions shift substantially. The distribution matters because it protects against two errors: the error of treating a secure classification as an achievement, and the error of treating an insecure one as a pathology.

Continuity and change. Attachment classifications are moderately stable within the first years and less stable across longer periods; they change with the caregiving environment. A child in an unpredictable household can move from secure to insecure; a child moved to a stable placement can move the other way. The clinical implication is the opposite of the popular one: attachment is not destiny, and the practitioner's task is to change the environment, not to accept the classification.

Synchrony. The matching of interaction between caregiver and infant — gaze, vocalisation, touch, pace, timing. Synchrony is not perfect imitation; it is contingent responsiveness, a caregiver responding to what the infant actually did, at a pace the infant can follow. Its absence is the earliest observable sign of relational difficulty.

The still-face experiment. Tronick's procedure: the mother interacts normally, then holds a still face for two minutes, then resumes. The infant's response is a sequence: attempts to engage, then distress, then withdrawal, then — on the mother's return —

recovery. The experiment is the strongest available demonstration of two things: that infants are active participants in interaction, and that repair after rupture is what matters, because the infant recovers rather than concluding that the relationship is broken.

Rupture and repair. Healthy development does not require constant attunement — no caregiver achieves it — but frequent enough attunement and effective repair. The concept is the single most useful in this week for family therapy, because it applies at every age: the goal is not the absence of rupture but the capacity to repair, and every family the therapist sees is organised around an unrepaired rupture somewhere.

Co-regulation. The borrowed regulation of early infancy: the caregiver's voice, body, rhythm and face down- regulate the infant's arousal, repeatedly, and the infant gradually internalises the pattern. Self-regulation is therefore a relational achievement before it is an individual capacity, which is why the advice to "let him cry it out" is a technique imposed on a relationship rather than a developmental fact.

What family therapists do with this. Three uses. First, an assessment frame: what strategy has this child developed, in this household, and what does it protect them from? Second, a formulation of the parents' own strategies: their working models are in the room, and their responses to the child's distress are shaped by what happened to them. Third, a target: the repair, not the classification — one reliably repaired rupture is worth more than a theory of the child's pattern.

3.2a Four infants, described and classified

The classification skill, practised on four descriptions, with the reasoning made explicit. Students should classify each before reading the analysis.

Infant one. Eleven months. On separation, cries briefly and looks at the door. On reunion, reaches up, is picked up, settles within thirty seconds, and then struggles to be put down so that she can return to the toys. She shows the stranger a toy from the safety of her mother's lap.

Classification: secure. Reasoning: distress at separation, active greeting, effective use of the caregiver for comfort, and a return to exploration. The critical feature is not the distress but the use of the caregiver.

Infant two. Eleven months. On separation, continues to play and does not look at the door. On reunion, does not approach, turns away, and continues with the toys; when the caregiver picks him up, he arches and looks over her shoulder. The observer notes that his heart rate rose during the separation.

Classification: insecure-avoidant. Reasoning: the absence of signalling, the avoidance at reunion, and the physiological arousal that the behaviour conceals. The strategy is the point: not signalling is a strategy, learned from a caregiver for whom signalling has not worked.

Infant three. Eleven months. On separation, is inconsolable and cannot be distracted by the stranger. On reunion, clings, cries, is picked up, and then pushes against the caregiver while continuing to cry; is not settled within the twenty minutes.

Classification: insecure-resistant (ambivalent). Reasoning: the amplified signalling, the failure to be comforted, and the mixture of contact-seeking and contact-refusing. The strategy is to keep the attempt alive, because settling would mean that the availability is settled, and the infant's experience is that it is not.

Infant four. Eleven months. On separation, stands still and does not cry. On reunion, approaches the caregiver with his head turned away, raises a hand to his mouth, and then freezes for several seconds before walking in a circle. Twice he begins to approach and stops.

Classification: disorganised. Reasoning: contradictory, incomplete and undirected behaviour; freezing; no coherent strategy. The clinical action is not a classification but an assessment: what is happening in the household, and is this infant safe?

What the four descriptions teach. That the classification depends on the reunion, not the separation; that behaviour and physiology can disagree, which is why observation alone can mislead; that every classification is a description of a strategy; and that the fourth requires an action rather than a label.

The common error. Trainees classify infant two as secure because he is not distressed. The error comes from watching the separation. The correction is simple: watch the reunion, and ask what the child does with the caregiver when the caregiver comes back.

3.3 Synchrony, assessed in the room

A family therapist does not run a Strange Situation, and can assess the same processes in an ordinary session. Six observable markers, with the behaviour that indicates each.

Marker one — the greeting. When the caregiver returns from a brief absence (leaving the room to take a call), does the infant look up, vocalise, move toward? Does the caregiver acknowledge the infant on return? A pair who do neither, repeatedly, are telling the therapist something.

Marker two — the pace. Does the caregiver wait for the infant's response before speaking again, or fill every silence? Rapid, uninterrupted caregiver speech is the most common synchrony problem in anxious parents, and it is also the easiest to change.

Marker three — the repair. When the infant becomes distressed in the session — a bump, a fright, a stranger's approach — what does the caregiver do, and how quickly does the infant settle? The repair is the assessment, and it takes ninety seconds to observe.

Marker four — the use of the caregiver as a base. Does the infant explore the room, return to the caregiver, and go out again? The pattern of excursions and returns is the secure base in miniature.

Marker five — the caregiver's reading. Does the caregiver interpret the infant's signals accurately? "He is tired" when the infant is hungry is a small error; a caregiver who cannot read any signal is a different finding. Ask the caregiver to narrate: "what is he telling you now?" The narration is the assessment.

Marker six — the caregiver's own state. A depressed, frightened, exhausted or intoxicated caregiver cannot provide synchrony, and the observation should be recorded as a statement about the caregiver's state rather than about their character. This is the marker that most often produces the referral.

How the six are used. Observed in the first fifteen minutes of a session, recorded in behavioural language, and used as the basis for the formulation. The therapist does not score the pair; the therapist describes, and the description is the intervention, because it teaches the caregiver to look at what they are already doing.

The caution. Fifteen minutes in an unfamiliar room, with a stranger present, is not a representative sample. The observation is a hypothesis generator, not a finding, and the therapist should say so and ask the caregiver what the infant is like at home.

3.3a Co-regulation, designed for one family

The concept is easy to state and hard to apply. A worked sequence for a family in which the caregiver's own regulation is the problem.

The family. A mother, 24, with a nine-month-old, referred by the health visitor for "he cries all the time and I cannot cope". She is sleeping four hours a night, has no help, and describes herself as "shouting at a baby". Her own history includes a violent father and a mother who "never held us".

The formulation. The infant's crying activates the mother's own attachment system: she experiences the crying as an accusation, her arousal rises, and she responds with the only strategy she has — withdrawal, followed by shouting. The infant's crying escalates, the mother's arousal escalates, and the loop is complete within ninety seconds. Neither is doing anything pathological; both are doing what their systems have learned.

The co-regulation sequence, taught in the session.

Step one — the mother's own regulation, first. "Before you can calm him, you have to lower your own arousal, because he reads you. What is the fastest way for you to bring yourself down?" Her answer: cold water on her wrists, and counting to ten out loud. Both are taught and rehearsed in the room, with the baby present, so that she experiences the sequence rather than being told about it.

Step two — a holding position that works for her. Not the textbook position: a position she can sustain without feeling trapped. She chooses to sit on the floor with him against her chest, which is a position she can leave if she needs to, and which she can hold for two minutes.

Step three — a sound she can make. A low, repeated sound, not a song, because a song requires performance. She chooses "shhh" on the out-breath, which is also the physiological sigh that lowers her own arousal, so the technique regulates both of them at once.

Step four — the two-minute rule. She commits to two minutes of steps one to three before doing anything else. Two minutes is chosen because it is achievable when she is exhausted, and because most infant crying cycles change within two minutes.

Step five — the permission to put him down. "If after two minutes you still cannot, put him in the cot, leave the room, and stand at the door for three minutes. That is not abandonment; that is you preventing something worse." The permission is the intervention's ethical core: a mother who is frightened of her own anger needs a legitimate exit.

The follow-up. Two weeks later: the two-minute sequence is used "most times", the shouting has reduced, and the mother reports one occasion on which she put him down and stood at the door. She describes it as "the first time I did not feel like a bad mother". The infant's crying has not disappeared; the loop has been interrupted.

What the sequence demonstrates. Co-regulation is not a technique for the infant. It is a technique for the relationship, and it begins with the caregiver's own state — which is why this week's material belongs in a family therapy course and not only in a developmental psychology text.

3.3b The caregiving network, mapped

The single most useful correction to the dyadic literature, and the instrument that puts it into practice.

The instrument. A circle in the middle for the infant, and around it every person who does caregiving work, with three pieces of information: how often, for how long, and what they do. The answers are usually wider than the referral: a grandmother who takes the nights, an aunt who takes the child at weekends, an older sibling who walks him to nursery, a neighbour who minds him while the mother sells.

Why it matters clinically. Three reasons. First, the assessment of "the mother–infant relationship" is incomplete and often unfair when four people are raising the child; the mother is graded on a job she is doing with help, or failing to do because the help is contested. Second, the infant's attachment strategies are distributed across the network: a child may be secure with the aunt and avoidant with the father, and the classification literature's single-figure procedure cannot describe this. Third, the network is where the intervention usually belongs: the contested rule, the inconsistent method, the caregiver who undermines another, the person who is exhausted.

The three questions that map it. "Who has held him this week, and for how long?" "Who decides what happens when he cries?" "Who disagrees with whom about how he should be raised?" The third question is the clinical one, and it is almost never asked in a developmental assessment, although it explains more of the household's difficulty than any milestone.

A worked map. The Balogun infant of Week 2: mother (all nights, most days), father (rest days, evenings), paternal grandmother (four nights a week, all cooking, some feeds), an aunt (Saturdays), and a neighbour (two afternoons a week while the mother works). Decisions: the grandmother decides feeding and bathing; the mother decides sleep; nobody decides anything about the crying, which is where the disputes are. Disagreement: the grandmother believes the infant should be left to cry; the mother cannot bear it; the father follows whoever is in the room.

The intervention the map produces. Not a parenting programme. One conversation with the three adults about crying — what they each do, what they each believe, and one agreement they can all keep — plus the assignment of one decision to the mother, explicitly, so that she has an area of authority. The map made the problem visible: the household was not failing to respond; it was responding four different ways.

The caution. Mapping the network can expose a conflict that the family have managed by not discussing it, and the therapist should not conduct the exposure in a first session with the infant present. Map it, note it, and bring the adults together separately when the map is complete.

3.4 The first session with an infant and two parents

An assessment structure for the youngest client, in five phases, with the observation built into the session rather than bolted on.

Phase one — the frame (five minutes). "I am going to ask about the pregnancy, the birth and the first months, and then I will spend some time just watching the three of you together, because that tells me more than any question. You do not need to perform anything; do what you would normally do." The permission not to perform is necessary, because parents of infants believe they are being graded.

Phase two — the chronology (fifteen minutes). The pregnancy, the birth, the first weeks, feeding, sleep, who has helped, and what has changed in the household. The timeline is drawn as in Week 1, with one addition: the parents' own events — a job loss, a bereavement, a move — are marked on the same line as the infant's, because the two are one story.

Phase three — the observation (fifteen minutes). The therapist stops asking questions and watches. The six markers of 3.3 are noted silently: greeting, pace, repair, use of the caregiver as a base, the caregiver's reading, the caregiver's own state. The therapist says nothing for the first five minutes, which is uncomfortable for everyone and is the most informative part of the session.

Phase four — the narration (ten minutes). "Can I tell you what I noticed? You waited for him to look at you before you picked him up, and when the door banged he looked at you before he cried. That is a child who checks with you." The narration is in behavioural language, it names what is going well first, and it is specific. The parents' response to the narration — relief, tears, argument — is itself data.

Phase five — the formulation (five minutes). One of the five statements from Week 1, plus one thing to try before the next session. The thing to try is almost always relational and small: a two-minute co-regulation sequence, a pause before responding, a handover of one caregiving task between the parents.

What the therapist does not do. Does not test the infant; does not classify the attachment; does not interpret the parents' childhoods in session one; does not give feeding or sleep advice unless asked, and then with the evidence stated plainly in both directions.

3.4a The still face, in the consulting room

The experiment cannot be replicated with a family, and its lesson can be demonstrated, and the demonstration is the week's most powerful teaching device.

The demonstration. With a volunteer parent and infant (or a peer and a doll), the therapist asks the parent to play normally for one minute, then to hold a still face for thirty seconds, then to resume. Most parents find the thirty seconds distressing, and most infants mount a visible sequence of attempts, protest, and withdrawal within twenty seconds. The debrief is the teaching.

What the demonstration shows. That the infant is not a passive recipient but an active partner with strategies; that the infant's distress is proportionate and immediate; that the resumption produces recovery in most pairs; and that the parent's own discomfort during the still face is a reliable index of how much the interaction matters to them.

The clinical use. The demonstration is used with parents who believe their infant does not notice them — fathers who feel irrelevant, mothers who believe the baby prefers the grandmother, parents who have been told that "at this age they do not know anything". Showing a parent that their infant is tracking them is more persuasive than any amount of explanation, and it changes the parent's behaviour within the session.

The caution. Never with a distressed or ill infant, never without consent, never for longer than thirty seconds, and never as a test of the parent. The demonstration's purpose is the parent's discovery, not the therapist's evidence.

3.5 Case: the Adeyemi family, in the infant's terms

The course's recurring family, now read for the infancy that shaped it — because the infancy is where the patterns the family present were built.

What we know. Amara is fourteen; Segun is eleven. Their father, Tunde, has worked away for most of their childhood. Their mother, Funke, has managed the household alone. The paternal grandmother lived with them until her death in 2023.

The reconstruction, offered as a hypothesis and not as a finding. Funke's account: Tunde was present for the first three months of Amara's life, then took the contract in Lagos. She describes those three months as "the best time" and the following year as "the worst". She returned to work at six months and left Amara with her mother-in-law, with whom she "did not agree about anything but could not say so".

The attachment reading. Amara's first year contained two discontinuities: her father's departure at three months, and her mother's return to work at six with a substitute caregiver whose methods her mother could not endorse. Neither is a trauma; both are the ordinary material from which a working model is built. The hypothesis — and it is a hypothesis — is that Amara learned that the person who is most available is also the person whose authority is contested, and that need is best managed by being self-sufficient and by keeping the adults' attention through achievement and, later, through refusal.

Segun's first year, by contrast. Funke did not return to work until Segun was two, and the grandmother was already ill, so Funke was at home. Segun's strategy is different: he stays close, he manages the household's information, and he is the family's listener. The difference between the siblings is a natural experiment in caregiving conditions, and the family's own account confirms it.

What the therapist does with the reconstruction. Not much, and carefully. The reconstruction is used for two purposes: to understand why the mother's pursuit and the daughter's withdrawal are so well rehearsed, and to ask one question that opens the history without demanding it — "when Amara was small, who held her?" The question is asked once, in the presence of both parents, and whatever comes back is received.

The caution. Retrospective reconstruction of infancy is a hypothesis, not a record; memory is unreliable and the family's account is shaped by the present. The therapist who presents a reconstruction as a finding has replaced the family's story with a theory, which is the error Week 11 of MFT 702 warns against. The reconstruction earns its place only if it produces a question the family can use.

3.6 Annotated transcript: rupture and repair in a session

Ten turns from a first session with a mother (26) and her eleven-month-old, referred for "he will not settle with anyone but me". The transcript shows a repair happening live.

> **T1 Therapist:** He is looking at the box of toys. What does he usually do in a new room?
> **T2 Mother:** He clings. He will not go.
> **T3 Therapist:** (waits) He has let go of your sleeve.
> **T4 Mother:** (looking) He has.
> **T5 Therapist:** What would happen if you sat on the floor and did not reach for him?
> **T6 Mother:** He will cry.
> **T7 Therapist:** Let us find out. I will be quiet.
> **T8 Mother:** (sits on the floor; the infant moves two steps, looks back, and she reaches for him) — see?
> **T9 Therapist:** He looked back first. That is what he does before he decides. If you wait for the second look, he might go further. Would you try it once more?

> **T10 Mother:** (sits still; the infant moves four steps, looks back twice, she stays; he brings a block back to her) — he brought it to me.

Annotations. T1 begins with the child rather than the complaint, and asks the mother for her prediction, which is the assessment of her reading.

T2 is the maternal expectation — "he clings" — which is both accurate about the past and a prediction that will organise the future. The therapist does not challenge it; she waits.

T3 is the noticing, in behavioural language, of something the mother has not seen because she is answering a question. The therapist's silence at the start of the turn is doing work: it gives the infant time to act.

T5 is the invitation, framed as an experiment rather than an instruction, which is what makes it acceptable. T7 explicitly removes the therapist from the interaction, which lowers the pressure and makes the mother the agent.

T8 is the rupture: the mother reaches for the infant at the first look, which is precisely the pattern that prevents exploration. Her "see?" is not defiance; it is the proof of her own prediction.

T9 is the repair, and it is the transcript's most skilful move. The therapist does not say "you are over-responsive"; she names the mechanism ("he looked back first; that is what he does before he decides") and offers a small modification ("wait for the second look"). The mother's prediction is not contradicted; it is given a condition.

T10 is the outcome, and it is the mother's sentence — "he brought it to me" — not the therapist's. The intervention was ninety seconds long, and it changed a working model in the room.

What the trainee should notice. The therapist made no interpretation about the mother's own childhood, gave no advice about independence, and did not celebrate the outcome. The technique is a wait, a noticing, and an invitation, and it requires the therapist to tolerate the mother's distress at T8 without rescuing either of them.

3.7 Laboratory: observing synchrony

Sixty minutes, in threes, with a video clip and then a live observation.

Task one — the clip (twenty minutes). A consented three-minute clip of a caregiver and infant. First five minutes: description only, in behavioural language, no interpretation. Then the six markers, noted. Then one rupture and one repair, described.

Task two — the live observation (twenty-five minutes). One student brings a recording (or plays a parent with a doll); the therapist observes for five minutes in silence and then narrates what they saw, beginning with what is going well. The observer checks whether the narration was behavioural and whether it named a strength first.

Task three — the co-regulation design (fifteen minutes). For the family in 3.3a, design the five-step sequence and rehearse step one out loud.

Marking rubric.

Criterion	Weight	Full marks
Behavioural description	25%	No interpretation for the first five minutes; behaviour, timing and sequence named
Marker identification	20%	All six addressed, including the caregiver's own state
Rupture and repair	20%	One of each identified and described in behavioural language
Narration	20%	Strength named first; specific; the parent could repeat it
Co-regulation design	15%	Begins with the caregiver's regulation; achievable; an exit included

The debrief question. "What did you want to interpret, and what stopped you?" The answers are usually about the parents' own histories, and the discussion of why a first session is not the place for that interpretation is the week's most useful ten minutes. The rule to take away: describe before you explain, and explain only what the family have already begun to say.

3.8 Six errors in infant mental health work

1. **The label as a finding.** "He is insecurely attached" said to a parent who came for help with sleep. Fix: describe the strategy, not the classification, and only where it serves the work.
2. **The mother as the patient.** The observation focused entirely on the mother, with the father, the grandmother and the household invisible. Fix: the caregiving network as the unit, and every caregiver observed.
3. **The interpretive leap.** "He clings because you are anxious" offered in the first session, before trust. Fix: describe, narrate, then hypothesise — in that order, over sessions.
4. **The technique imposed.** Sleep training, independent play, or "let him cry" prescribed without attention to the family's capacity, beliefs and circumstances. Fix: the evidence stated plainly in both directions, and the family's decision respected.
5. **The missed caregiver state.** A depressed, frightened or exhausted caregiver observed and not addressed, because the referral was about the baby. Fix: the caregiver's own state is the sixth marker, asked about and responded to.
6. **The ignored history.** A disorganised pattern observed and not followed up, because following it up would require asking about violence. Fix: ask, record, and act — the same screen as Week 2, applied to infancy.

3.9 Culture, faith and the first relationship

Where the week's material travels well. The behavioural-system account is one of developmental psychology's more portable ideas: every human infant signals, every caregiver responds or does not, and the resulting strategies are recognisable across cultures. The concepts of synchrony, rupture and repair are observable in any household, and the co-regulation sequence is a description of what most caregivers already do.

Where it strains, four ways.

The Strange Situation's assumption of the dyad. The procedure assumes a single primary caregiver, and it assumes that the infant's reaction to a stranger and to a brief separation is meaningful in the same way everywhere. In households where multiple caregivers are routine — grandmothers, aunts, older siblings, paid helpers — the infant's reaction to the departure of one figure is a different measurement. The literature on multiple attachments is clear that infants form several, and the clinical assessment should therefore map the network rather than grade the mother.

The value placed on exploration. The secure pattern's marker is a return to play, which encodes a cultural valuation of exploration and independence. A household that values

compliance and closeness will produce an infant who stays near the caregiver, and the observer who scores exploration as the healthy outcome has imported a value into the assessment. The clinical question is not "does he explore?" but "does he use his caregiver when he is frightened?"

The practice of separation. Some households do not leave an infant with a stranger, ever, and some leave the infant for months with a relative in another town. Both produce patterns the laboratory did not anticipate. The therapist's task is to understand what the separation meant in this family, and to ask about it rather than to infer from a classification.

The independence curriculum. Sleep training, feeding schedules and early independence are cultural practices presented in some of the literature as developmental norms. A family who sleep with their infant, feed on demand and carry the child are not failing a developmental task; they are following a different curriculum, and the therapist who treats the curriculum as the problem has mistaken a practice for a milestone.

Faith and the first relationship. Naming ceremonies, blessings, dedication, prayer over a newborn and the forty-day lying-in period are all attachment-relevant events: they mark the child's entry into a community that will share the caregiving. The therapist should ask who was present, what was done, and who has a claim on the child's care as a result. The answers frequently reveal a caregiving network that the referral had rendered invisible, and in some families they reveal a dispute about the child that is the real presenting problem.

The examinable sentence. "Attachment is a strategy learned in a particular caregiving network, and the clinical task is to describe the strategy and the network, not to grade the pair."

> **DEBATE**
> "The Strange Situation measures a relationship, and using it to classify a child is a category error." Argue both sides, and say what the classification is useful for.

3.10 Evidence note: strong theory, moderate evidence, popular misuse

What is well established. That infants organise attachment behaviour and that individual differences can be reliably classified; that maternal sensitivity predicts security, with a moderate association; that classification is moderately stable in the early years; that disorganised classification is associated with frightening or frightened caregiving and with later difficulties; and that early security predicts later social functioning, weakly to moderately, with the effect mediated by later experience.

What is contested or weaker than commonly claimed. The strength and specificity of the link between infant attachment and adult attachment style — the transmission literature is real and the effect sizes are modest; the predictive power of attachment classification for later psychopathology, which is small in low-risk samples; the claim that the first two years determine later relationships, which the evidence does not support;

and the clinical usefulness of attachment classifications in ordinary family work, as against the description of strategies.

What the popular literature gets wrong. Three things, all of which families bring into the consulting room. That attachment is love, so that a "securely attached" child is a loved child and an "insecurely attached" one is not. That attachment is fixed, so that a difficult first year is a life sentence. And that attachment is a parenting technique — baby-wearing, co-sleeping, feeding — rather than a property of the relationship's reliability. Each of the three is a distortion, and each is common enough that the practitioner should be able to correct it in one sentence.

What the practitioner may claim. "Infants develop strategies for getting help from the people who care for them; those strategies are describable; they are shaped by the caregiving they receive; they change when the caregiving changes; and the repair of ruptures matters more than the avoidance of them." Each claim is defensible, and together they are enough for clinical work.

3.11 Key terms

Term	Definition
Attachment behavioural system	The organised behaviours a child uses to maintain proximity to a caregiver under threat
Internal working model	The child's expectations about self, others and help, built from repeated experience
Strange Situation	Ainsworth's laboratory procedure, classified on reunion behaviour
Secure (B)	Signals used, caregiver effective, exploration resumed
Avoidant (A)	Signalling suppressed, caregiver avoided at reunion, arousal often elevated
Resistant/ambivalent (C)	Signalling amplified, contact sought and refused, difficulty settling
Disorganised (D)	Contradictory, incomplete or undirected behaviour; no coherent strategy
Synchrony	Contingent, paced responsiveness between caregiver and infant
Still face	Tronick's procedure demonstrating infant participation and the importance of repair
Rupture and repair	The ordinary cycle of mismatch and restoration that builds resilience
Co-regulation	The borrowed regulation of arousal that precedes self-regulation
Caregiving network	All those who care for the infant, not only the parents

3.12 Self-test

1. What is an attachment classification a classification of?
2. Why does the reunion, rather than the separation, determine the classification?
3. Describe the strategy of the avoidant infant and the caregiving that produces it.
4. What makes disorganised attachment a red flag rather than a category?
5. State the relationship between synchrony, rupture and repair.

6. What does the still-face experiment demonstrate, and how is it used clinically?
7. Why is co-regulation a relational achievement?
8. Name three ways the popular literature misuses attachment, and correct each in one sentence.

Answers. 1. Of the strategy a child has developed for obtaining help from a caregiver under stress. 2. Because the strategy is revealed in what the child does with the caregiver when the caregiver returns — the separation shows distress, the reunion shows the organisation. 3. The avoidant infant suppresses signalling and avoids the caregiver, while physiology often shows arousal; the caregiving is typically rejecting of attachment behaviour, particularly distress. 4. Because it indicates contradictory or absent strategy, is associated with frightening or frightened caregiving, and requires an assessment of safety rather than a label. 5. Synchrony is frequent enough attunement; ruptures are inevitable; repair after rupture is what builds the child's confidence that the relationship survives. 6. That infants are active participants who respond to unresponsiveness and recover when responsiveness resumes; clinically it shows parents that their infant is tracking them. 7. Because the capacity to calm is first borrowed from a caregiver's voice, body and rhythm, and only later internalised.

1. That it is love (it is a strategy); that it is fixed (it changes with caregiving); that it is a technique

(it is a property of the relationship's reliability).

3.12a What to tell a parent in one page

The clinical material of this week, converted into something a parent can take home. Six sentences, in ordinary language, each with the developmental idea behind it.

1. **"He is not manipulating you; he is asking you for something, and he only has one way of asking."** The behavioural system is a communication system, and the infant's signals are the whole of the vocabulary.
2. **"You do not have to get it right every time. You have to get it right often enough, and come back after the times you miss."** Rupture and repair: the repair is what the infant is learning about, not the perfect attunement.
3. **"When you calm him, you are lending him your calm. Later he will do it himself, because he has borrowed it from you a thousand times."** Co-regulation as the precursor of self-regulation.
4. **"He checks your face before he decides how to feel about things, so your face is part of how he learns to feel."** Social referencing, and the reason a parent's own state is part of the child's environment.
5. **"If you are frightened, he will be frightened, and that is not your fault; it is the reason we are working on your situation as well as his."** The sixth marker, and the reason the assessment is of the household.

6. **"There is no single right way to raise a child in this family; there is the way you and the people who help you can agree on, and the agreement is what he needs."** The caregiving network, and the reason the therapist maps it.

How the page is used. Read aloud in the session, in the family's language, with the parent's own examples substituted for the general statements. Handed over at the end, with the therapist's name and the next appointment — not as a leaflet, but as a record of the conversation the family has just had.

3.13 Reflective task

Watch a caregiver and an infant for fifteen minutes in a public place — a bus, a clinic waiting room, a market. Describe, in writing, three things: one moment of attunement, one rupture, and one repair if there was one. Do not interpret, and do not judge. Then write two sentences on what it felt like to watch without interpreting. Most students find the third instruction the hardest, and the discomfort is the week's lesson.

3.14 Readings for this week

- **Bowlby (1969)**, Attachment and Loss, Vol. 1, the founding statement, read the chapters on the behavioural system.
- **Ainsworth et al. (1978)**, Patterns of Attachment, for the Strange Situation and the caregiving correlates.
- **Tronick (2007)**, The Neurobehavioral and Social-Emotional Development of Infants, for the still face and the repair literature.
- **Sroufe (1996)**, Emotional Development, for the longitudinal account of continuity and change.
- **Main & Solomon (1990)**, on disorganised attachment, read with the caution the concept requires.
- **Keller (2007)**, Cultures of Infancy, the essential corrective on the dyad and the caregiving network.

Read Keller before Ainsworth; the corrective first, so that the classification is read as a description of one kind of caregiving arrangement. For the case analysis, use at least one source that disputes the predictive power of attachment classification, and say what it means for your formulation. The reading-log question this week is: "what does attachment language let me see, and what does it stop me seeing?"

Next: Week 4 — Toddlerhood: autonomy, language and limits

3.2b The four patterns in adult life: what the family therapist sees

The infant patterns are the course's foundation for later work, and the bridge to the adult literature is worth building explicitly, because it is what the families' own patterns look like in a session.

Pattern	In the infant	In the adult, under stress	What the therapist sees in a couple
Secure	Signals, is comforted, returns to play	Seeks support, tolerates the answer, negotiates	Asks for what they need, hears the answer, can repair
Avoidant	Suppresses signalling, avoids at reunion	Manages alone, dismisses the need, works	Withdraws, gives facts, resists affect, seems calm
Resistant	Amplifies signalling, hard to settle	Escalates, monitors, cannot be reassured	Pursues, protests, cannot register the reassurance offered
Disorganised	No coherent strategy; freezes	Approach and withdrawal in the same minute	Contradictory moves, sudden coldness, intense pursuit then flight

Why the bridge is useful and dangerous. Useful because it gives the trainee a vocabulary for the couple's pattern that connects to a child's experience rather than to a personality type. Dangerous because the adult categories are not the infant classifications, and the popular habit of describing a partner as "avoidant" or "anxious" collapses a strategy under stress into an identity. The therapist's language should keep the condition: "under stress, you manage alone; under stress, you escalate" — not "you are avoidant".

The clinical use in three sentences. "When your wife raises her voice, you go quiet and give facts, and she escalates because the quiet looks like indifference. Both of you are doing what you learned to do when you were frightened, and by the time you are in this room both of you are frightened. The work is to change what happens in the ninety seconds after the fear, not to change who you are." That paragraph is the bridge, and it is recognisably Week 7 of MFT 702 in developmental clothing.

The three cautions. First, do not diagnose attachment styles from a session; the pattern you observe is a pattern under the specific stress of being observed. Second, do not use the vocabulary as a verdict — "he is an avoidant type" — because it does what the classification was designed not to do. Third, do not explain the parents' childhoods to them in the first months of therapy; the reconstruction is a hypothesis, and it is asked about, not delivered.

3.7a The disorganised flag, and what followed

A case in which the fourth pattern appeared in a session and required an action rather than a formulation.

The referral. A mother (29) and her fourteen-month-old, referred by a community nurse for "he is frightened of her". The mother describes the child as "always crying" and says she has "lost her patience". She is reluctant to come and says the nurse "made her".

The observation. In the first twenty minutes the infant plays on the floor, and twice, when his mother stands up to reach for a bag, he freezes with his hand at his mouth. When she sits again, he approaches to within a metre and stops. When she speaks sharply to the therapist, he stops playing entirely and watches his mother's face.

The therapist's internal response. A wish to reassure the mother, and a competing wish to protect the child. Both are data. The therapist notes them and does not act on either in the session.

The action, in the session. The therapist does not interpret, does not classify, and does not ask about violence in front of the infant. She ends the session ten minutes early, with a practical frame ("I would like to see you again this week, on your own, before we meet with him again"), and she records the observation in behavioural language: "infant froze twice on maternal movement; hand-to-mouth; stopped play on maternal sharp speech".

The individual session. Asked directly and privately: "is there anyone in the house who frightens you?" The mother cries and describes a partner who shouts and who has pushed her twice, "but not near the baby". The therapist does not accept the qualification as reassurance and does not argue with it. She names the risk to the infant plainly, once: "a baby who freezes when you stand up is a baby who has learned that movement can be dangerous. That is not your fault and it is not nothing."

The plan. A safety plan for mother and infant; referral to a domestic abuse service with the mother's agreement; a discussion about whether the partner should be seen, and a decision that he should not, for now; and a report to the safeguarding lead, explained to the mother in advance, in plain language, with the reason.

What happened. The partner left the household after a further incident. The mother and infant were seen for four months. The freezing at maternal movement disappeared within six weeks; the mother's own account of her history — a violent father, a mother who "never held us" — was the subject of the middle phase of the work.

What the case teaches. That the fourth pattern is a signal that requires an action, not a category to be recorded; that the observation must be written in behavioural language if it is to be used; that the screen is asked privately and the answer acted on; and that the therapist's own wish to reassure is a warning sign, because it is usually the wish to avoid what the observation has already shown.

Week 4 — Toddlerhood: autonomy, language and limits

Developmental tasks. Autonomy versus shame and doubt (Erikson); language explosion (from ~50 words at 18 months to 300+ and two-word combinations by 24 months); symbolic play; beginning of self-regulation.

The clinical translation of "no". A toddler's "no" is the emergence of a separate self. The parental task is to hold the limit without humiliating the child — "You may not hit. You may be angry." This is the earliest training ground for the emotional vocabulary the family will use for the next fifteen years.

Language and family. Delay in expressive language is the most common developmental concern presented to clinics. Distinguish: late talker (normal variation with good comprehension), bilingual language confusion (normal), hearing loss, global delay, and

autism spectrum presentation. Always ask about **hearing** and about **which language is spoken at home and by whom**.

> **CASE**
> A grandmother raises a 2-year-old while both parents work. She reports he "does not listen". Assessment finds: he is spoken to mostly in commands, in two languages, by three adults with different rules. The intervention is not a behaviour chart; it is a family agreement about one set of five rules, one language of instruction, and ten minutes of child-led play a day with each caregiver.

4.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The second year	What changes, and why it is the hardest year for most households	Name the toddler behaviour they find hardest
10–25	Autonomy and shame	Erikson's pair, and the difference between the two outcomes	Rewrite three shaming sentences
25–40	"No" as a developmental act	Three recorded "no"s at 18 months, 24 months, 3 years	Classify each and predict the parent's best move
40–55	Language	The vocabulary explosion, and the five-way differential for delay	Take a language history
55–70	Limits without humiliation	The formula, demonstrated on three scenarios	Rehearse the formula in pairs
70–82	The household's rules	Why three caregivers and five sets of rules is the commonest presentation	Map one household's rules
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. This week's material is where the course most often becomes advice, and advice is what the families have already had. The lecture should therefore take the commonest presentation — "he does not listen" — and demonstrate that the answer is not a technique but a household agreement, arrived at by mapping who says what to whom. The class should leave with the habit of asking, before any behavioural advice, "who else is giving him a different rule?"

The sentence the lecture must land. "The toddler's task is to become a separate person, and the family's task is to survive that without either crushing it or abandoning the limits." Every technique this week is judged by whether it serves both halves of that sentence.

4.2 Core concepts

Autonomy versus shame and doubt. Erikson's second stage. The toddler's task is to establish a self that can act — to walk, to choose, to refuse, to do it myself — and the outcome depends on whether the environment permits the acting. Too much control, or ridicule of the attempts, produces shame and doubt; too little produces a child without limits, who is also without safety, because a limit is a form of being held. The stage's clinical value is that it makes the tantrum intelligible: the child is not testing the parent's patience but the boundary of the self.

The developmental function of "no". The word appears at around eighteen months and peaks in frequency in the second year. Its function is to establish separateness: the child discovers that a word can stop an adult, which is the first experience of agency. The clinical implication is that a household which cannot tolerate "no" will produce either a compliant child or a child who escalates, and both presentations arrive in clinics.

The language explosion. From roughly fifty words at eighteen months to three hundred or more and two-word combinations by twenty-four months, with a further acceleration in the third year. Comprehension precedes production, which is the single most useful fact in the differential: a child with poor production and good comprehension is a different clinical case from a child with poor comprehension.

The five-way differential for language delay. A late talker (normal variation, good comprehension, family history); a bilingual or multilingual child (normal, and often assessed in the wrong language); hearing loss (the first thing to exclude, always); global developmental delay (delay in all domains); and an autism spectrum presentation (delay with differences in social communication, play and flexibility). The differential is decided by a hearing test, a comprehension assessment, a play observation, and a language history — not by the number of words.

Symbolic play. The capacity to use one object for another — a block as a phone — which appears in the second year and is the foundation of imagination, narrative and self-regulation. Its absence or its poverty is a clinical sign, and its presence is a resource the therapist can use.

Self-regulation, borrowed and then owned. The toddler's capacity to manage frustration is being assembled from the caregiver's co-regulation (Week 3) and from the language the family provides. The child who is told "you may be angry, you may not hit" is being given a vocabulary and a limit at once, which is the formula this week teaches.

Limits without humiliation. The formula has three parts: name the feeling, hold the limit, offer the alternative. "You are angry that we are leaving. We are not staying. You may hold my hand or walk beside me." The formula is not a trick; it is a structure that prevents the two failures — the limit without the feeling (which teaches that feelings are not allowed) and the feeling without the limit (which teaches that the world will accommodate anything).

The consistency problem. Toddler behaviour is shaped by consistency, and consistency in most households is not a matter of willpower but of coordination: three caregivers, five sets of rules, two languages, and a grandmother who does not accept the parents'

authority. The clinical presentation "he does not listen" is, in a majority of the families a therapist will see, a presentation of an uncoordinated household.

The one-language question. In multilingual households, the assessment question is not "which language is best?" — the evidence supports multilingualism — but "which language does each caregiver speak to the child, and is there a language in which the child hears sustained, contingent talk?" A child who hears commands in three languages and no conversation in any is at risk not of bilingual confusion but of language poverty.

Toddlerhood as the family's first discipline negotiation. Before the second year, the household's rules are about care; after it, they are about conduct. The negotiation of who has authority, whose family's methods prevail, and what happens when the child refuses is the household's first real conflict about child-rearing, and in many couples it is the first time they discover that they were raised differently.

4.2a The limit formula, taught and rehearsed

The week's core skill, in three parts, with the sentences and the failures.

The three parts. Name the feeling: "you are angry that we are leaving the park." Hold the limit, in a sentence with no argument in it: "we are leaving now." Offer the alternative: "you can walk to the car or I can carry you."

Why the order matters. The feeling comes first because a child in distress cannot process a limit until they have been understood; the limit comes second because a limit offered without the feeling teaches that feelings are inadmissible; the alternative comes third because a child needs something to do with the refusal, and a choice is the smallest available autonomy.

Three rehearsals.

Scenario one — hitting. "You are angry with your brother. You may not hit. You can stamp your foot and tell me, or you can sit here with me until you are ready." Note the limit is absolute (no hitting), the feeling is allowed, and the alternatives are two behaviours the child can actually perform.

Scenario two — the supermarket. "You want the sweets. We are not buying sweets today. You can help me find the rice, or you can sit in the trolley." Note that the second alternative is not a reward; it is a structure.

Scenario three — bedtime. "You do not want to stop playing. It is bedtime. You can choose the book, or I will choose it." Note the choice is real but bounded, which is the definition of autonomy within a limit.

The four failures.

The lecture. "You know you should not hit, we have talked about this, why do you always…" — the child is two; the lecture is for the parent's anxiety, and it lengthens the episode.

The bargain. "If you stop crying I will give you the phone." — the limit has been replaced by a price, and the child has learned that crying is a currency.

The threat that cannot be executed. "If you do not come now I will leave you here." — the child learns that the parent's words are not reliable, which is the opposite of what a limit is for.

The shame. "Big boys do not cry. Look at you." — the feeling is prohibited and the self is judged, which is exactly the outcome Erikson's stage warns against.

The tone. The formula is delivered quietly and slowly, without explanation, and repeated in the same words if it fails. The most common error after the words is right is the tone: a formula delivered at speed and volume is a reprimand with better vocabulary.

The repetition rule. The same three sentences, in the same words, every time the same situation recurs. Toddlers learn from pattern, not from explanation, and the parent who varies the wording is teaching the child that the limit is negotiable.

When the formula does not work. If the child escalates regardless, the limit is held with the body and with silence: stay, do not argue, do not leave, and wait. The episode ends. The parent's task is to survive it without adding a humiliation, which is what the formula is for.

4.2b The tantrum, anatomised

Tantrums are the second year's signature event, and the literature on them is more useful clinically than the advice most families receive.

What a tantrum is. A loss of the child's regulatory capacity under a combination of frustration, fatigue, hunger and an audience. It is not manipulation — manipulation requires a theory of the other's mind and a capacity to inhibit an impulse, both of which are under construction — and it is not a failure of parenting. It is the developmental task colliding with an immature nervous system.

The four phases, and what each needs. The trigger: a want blocked, a transition, a hunger, a tiredness, an over-stimulation. What it needs: anticipation, and fewer transitions when the child is tired. The escalation: shouting, going rigid, throwing. What it needs: safety, and the adult's calm — a co-regulation problem, not a discipline problem. The peak: the child cannot hear anything. What it needs: presence and very few words; this is the phase in which talking makes it worse. The recovery: the child returns, often clingy. What it needs: the repair — a cuddle, a naming of what happened, and no lecture.

The three errors at each phase. At the trigger: begging, bargaining, or giving in, all of which teach the child that escalation works. At the peak: reasoning, threatening, or shaming, all of which add a second distress to the first. At the recovery: a lecture, a punishment, or a demand for an apology, all of which lose the repair.

The one useful sentence at the peak. "I am here. I will wait." Spoken quietly, repeated, with no argument attached. The sentence is not a technique for stopping the tantrum; it is a technique for surviving it without damage, and the tantrum ends when the nervous system is finished, which is usually within ten minutes.

The household's contribution. Tantrums are more frequent in households with inconsistent rules, more transitions, more caregivers with different responses, and more

adult stress. The clinical implication is the week's: the frequency is a property of the household's coordination as much as of the child. That is not a criticism of the parents; it is the reason the intervention is a meeting about five rules rather than a lecture about the child.

What to tell the parents, in three sentences. "This is the year his feelings are bigger than his ability to manage them, and he needs you to be the calm one. When it peaks, talk less and stay. Afterwards, when he comes back to you, that is the moment that matters, not the tantrum."

4.3 Language: the assessment

The most common developmental concern in clinics, and the one with the clearest assessment pathway.

Step one — hearing. Always first, always. A child with glue ear or a sensorineural loss will present as a late talker, and no amount of language stimulation will change it. The referral is to audiology, and it is made in the first session in which language is the concern.

Step two — the language history. Which languages, spoken by whom, how much, and in what register? Does the child hear sustained talk — conversation, narration, questions — or mostly commands? Who reads to the child, and in what language? Is there a family history of late talking?

Step three — comprehension before production. "Show me the one that is bigger." "Give the ball to the grandmother." Comprehension is assessed by instruction, not by vocabulary, and it is the variable that most changes the differential: poor production with good comprehension is a different case from poor comprehension.

Step four — the play observation. Ten minutes of child-led play (Week 5's skill, used early): does the child use symbols, take turns, follow the adult's point, share attention on an object? The observation answers questions the vocabulary count cannot.

Step five — the settings. Does the child speak at nursery and not at home, or the reverse? A child who is fluent in one setting and silent in another is presenting a relationship or anxiety issue, not a language disorder — and the bilingual child who is silent at nursery is the commonest example.

Step six — the referral decision. Four outcomes: no concern (review in three months); monitor with specific advice (more talk, less command, one language of instruction where the household can manage it); refer to speech and language therapy; or refer to paediatrics or child development for a broader assessment. The decision is recorded with its reasons.

What the therapist says to the parents. Not "he is a late talker" and not "he may be autistic", but a statement of what is known and what will be done: "his hearing needs checking first; in the meantime, the most useful thing you can do is talk with him rather than to him — narrate what you are doing, wait for his sound, and answer it. We will look again in eight weeks."

The advice that works. Five practices, all supported by the language-development literature: follow the child's attention rather than directing it; narrate and expand ("yes, a big bus"); pause and wait for a turn; reduce questions and increase comments; and read the same book repeatedly. The fifth is the most enjoyable for families and the most effective per minute.

The advice that does not work. Flashcards, screens as language instruction, correcting the child's speech, and telling a bilingual household to abandon one language. Each is common, and each is either ineffective or harmful.

4.3a Ten minutes of child-led play, in the second year

The practice that appears again in Week 5, used early because it is the fastest way to change a toddler's language and a caregiver's relationship with the child at the same time.

The instructions to the caregiver. "Ten minutes, once a day, at a time when you are not rushed. He chooses what to play. You do four things and nothing else: you watch, you describe what he is doing, you copy what he does, and you say back what he says. You do not ask questions, you do not teach, you do not correct, and you do not use your phone."

Why each instruction is there. Watching teaches the caregiver to see what the child is actually doing, which is the skill the household lacks. Describing gives the child language attached to his own attention, which is the mechanism of vocabulary growth. Copying gives the child the experience of being followed, which is the antidote to a week of commands. And reflecting his speech back is the single most effective language intervention available to a non-specialist.

The prohibitions, and their reasons. Questions ("what colour is that?") test rather than converse, and the second year is not the age for testing. Teaching ("no, that goes there") converts play into instruction. Correcting ("say 'truck', not 'tuck'") interrupts the attempt at speech. And the phone, which is the most common failure of the practice in this decade, breaks the contingency the practice exists to build.

What the caregiver notices, in the first week. Three things, reported with surprise: that the child does more when not directed; that the child looks at them more; and that ten minutes is longer than they expected. The third observation is the one that changes behaviour, because it shows the caregiver that attention is a practice rather than a feeling.

What the therapist does with the reports. Asks for one specific thing the child did and one thing the caregiver did, so that the account is behavioural rather than evaluative. Then asks the question that generalises the practice: "what would it take to talk to him like that at other times of the day?" — which is where the language advice becomes a household habit rather than an exercise.

The contraindication. In households where the child's play is destructive or unsafe, or where the caregiver's own state makes ten minutes of undivided attention impossible, the practice is postponed and the caregiver's circumstances are the intervention. A parent who is being asked to do something they cannot do learns that they are failing, which is precisely what the second year's stage warns against.

4.4 Case: the grandmother who "cannot manage him"

The week's case, worked in full, because it is the presentation the course's own stub describes and the one that appears most often in clinics.

The referral. Tunde (2 years, 4 months), referred by a community nurse at the request of his grandmother, who says he "does not listen" and "shouts at me". His parents work: the mother (Yemi, 27) runs a stall six days a week; the father (Kola, 31) drives a taxi on shifts. The grandmother (58) has cared for Tunde since he was eight months old. The household also contains the father's younger brother (19).

The history. Walking at 15 months; first words at 14; two-word combinations at 27 months — at the lower end of range, in a household where three languages are spoken. Hearing has not been tested. Tunde attends a nursery three mornings a week, where he is described as "settled and quiet".

The three-setting comparison. At nursery: follows routines, speaks little, plays alone but near others. With his mother on Sundays: clingy, and speaks more, mostly in Yoruba. With the grandmother on weekdays: shouts, refuses, hits the furniture. Three settings, three children.

The rules map, which is the assessment's centre. The grandmother's rules: no playing outside, eat at the table, do not touch the radio, sleep at one. The mother's rules: outside is allowed in the yard, eating happens when he is hungry, the radio is not a problem, sleep at two if he is tired. The uncle's rules: none, and he gives sweets. The nursery's rules: a routine, a mat for stories, a rule about sharing.

The formulation, in three parts. First, Tunde is living in a household with four rule systems, two of which (the grandmother's and the mother's) contradict each other on almost every point. Second, the grandmother is doing the largest share of the care with the least authority: the mother's rules prevail in principle and the grandmother's in practice, and neither woman can say so to the other. Third, the shouting is the most effective strategy available to a two-year-old in this arrangement: it produces attention, and it produces a change of plan.

The language finding. Comprehension in Yoruba is good; production is delayed at the lower end of normal, in a trilingual environment. The referral for hearing is made in the first session. The language advice is not "speak one language" but "each caregiver speaks their strongest language to him, and everyone talks with him rather than at him" — which preserves the multilingual household and increases the child's exposure to sustained talk.

The intervention, in four parts. One: a family meeting with the three adults (grandmother, mother, father) in which the rules map is drawn and five rules are agreed — and only five, because five is a number a household can keep. Two: the authority question settled explicitly — the grandmother decides the weekday routine, the mother decides the weekend, and neither contradicts the other in front of the child. Three: the limit formula, taught to the grandmother first, because she is doing the work, and rehearsed with her in her own words. Four: ten minutes a day of child-led play, by whoever is with him, because it is the fastest way to increase his language and his experience of being followed rather than directed.

What the parents' absence means. The parents are not the caregivers during the week, and a plan that requires them to change their weekday behaviour will fail. The plan therefore works through the grandmother, and the parents' role is to endorse her authority in the child's hearing — one sentence, said in front of Tunde, once a week: "you do what Grandma says."

The outcome at eight weeks. The shouting reduced substantially; the grandmother reported that "he listens better", which in practice meant that she had stopped repeating herself and started using the formula; the hearing test was normal; the language was at age level in Yoruba and improving in English at nursery. The mother and the grandmother had one argument, in the third week, about the rules, and settled it without the therapist.

What the case teaches. That "he does not listen" is usually a statement about a household rather than a child; that the caregiver with the least authority is often doing the most work; that the intervention's site is the coordination between adults, not the child's behaviour; and that the language advice for a multilingual household is not subtraction.

4.5 Annotated transcript: the family meeting about five rules

Twelve turns from the family meeting in the case above, in which the therapist builds an agreement between three adults who have been contradicting each other for eighteen months.

> **T1 Therapist:** I want to make a list of what actually happens now, not what should happen. Grandma, what are your rules for him in the day?
> **T2 Grandmother:** No going outside. He eats at the table. He sleeps at one.
> **T3 Therapist:** (writing) And Yemi, when you are here on Sunday?
> **T4 Mother:** He can play in the yard. And he eats when he is hungry, I do not force him.
> **T5 Therapist:** (writing, then turning the sheet) So there are two sets, and both of you are doing what you think is right. Who decides when they disagree?
> **T6 Grandmother:** (pause) She is the mother.
> **T7 Mother:** (pause) But I am not here.
> **T8 Therapist:** So the person who is here has the rules of the person who is not here. That is very hard for both of you, and it is also very confusing for a two-year-old.
> **T9 Grandmother:** (nods) He knows. He goes to her when I say no.

> **T10 Therapist:** That is exactly it. He has learned that there are two answers. Can we agree five rules that are the same whoever is with him, and let everything else be each person's own business?
> **T11 Mother:** (to grandmother) Which five?
> **T12 Therapist:** (quietly) That is the question. You two decide, and I will write them down, and we will give the fifth to Kola when he comes.

Annotations. T1 asks for the actual, not the ideal, and addresses the caregiver with the most practice rather than the parents with the most authority. The choice is deliberate: it credits the work before it asks for change.

T2–T4 produce the two rule systems without conflict, because the therapist is writing rather than judging. The sheet is visible to both, which means neither woman is being asked to hold her rules in her head against the other.

T5 is the question the household has avoided for eighteen months, and it is asked as a question about the household, not about either woman's competence. T6–T7 are the answer, and the sequence is the finding: the grandmother concedes authority and the mother concedes presence, and neither had said so aloud.

T8 is the reformulation, and it is the session's turning point. It names the difficulty for both women, and it introduces the child's experience as the reason for change — which converts a power negotiation into a shared problem.

T9 is the grandmother's own evidence, offered freely once the conflict has been named, and it is better evidence than anything the therapist could have produced. The child "goes to her when I say no" is the two-year-old's solution to the household's problem, described by the person who has watched it.

T10 is the intervention: five rules, agreed, with everything else left as each person's business. The "five" is chosen deliberately — small enough to keep, large enough to cover safety — and the "everything else is your own business" is what makes agreement possible.

T11–T12 hand the authorship to the two women, and the therapist's last sentence adds the father without making him the absent problem. The fifth rule is "given to Kola", which enrols him and does not blame him.

What the trainee should notice. The therapist made no behavioural recommendation, gave no parenting advice, and did not name the grandmother's methods as wrong. She built a structure in which the household could agree, and the agreement — not the technique — is what changed the shouting.

4.5b The nine words a caregiver needs

A short section on the vocabulary of the second year, because the words are the intervention.

"You may not. You may be angry." The pair that separates the behaviour from the feeling, and the earliest lesson the child receives that both can be true.

"I will wait." The sentence for the peak of a tantrum: presence without negotiation.

"You choose." The smallest autonomy, offered where it is real: which cup, which book, which way to the car.

"Show me." The instruction that replaces "use your words" for a child who does not yet have them, and that teaches the caregiver to look for the message rather than the misbehaviour.

"That was hard, and you did it." The sentence that builds the child's account of himself as someone who manages difficulty, which is the second year's real prize.

"I am sorry I shouted." The sentence that models repair, and that most caregivers have never heard from an adult themselves. Its effect on a two-year-old is small and its effect on a household is large, because it teaches everyone in the room that repair is possible.

"What does he need?" The question the caregiver asks themselves instead of "why is he doing this?", and the one the therapist is trying to install.

"Who else says something different?" The therapist's question, not the caregiver's, and the one that turns a child's behaviour into a household's coordination problem, which is the difference between a week of advice and a change in the household's arrangements.

"We will do it the same way." The household's sentence, said by each caregiver to the child, and the smallest possible statement of an agreed structure.

How the nine are taught. Not as a list to be memorised, but one at a time, in the family's language, rehearsed in the session, and written on the page the family takes home. Three of the nine, chosen by the family, are enough for a fortnight; the rest come later, and some never do.

4.6 Laboratory: the formula and the rules map

Sixty minutes, in threes.

Task one — the formula under pressure (twenty minutes). Three scenarios, each played twice: first as a tired parent at the end of a day (the therapist plays the parent badly, on purpose), then using the formula. The observer records the three parts, the tone, and whether the parent explained.

Task two — the rules map (twenty-five minutes). Each student maps a household they know: who is present, what each person's rules are, where the rules contradict, and who decides when they do. Then write the question that would open the conversation, in the family's language.

Task three — the five rules (fifteen minutes). For the mapped household, draft five rules that all caregivers could keep, and mark which one is the safety rule. The test: could a grandmother, a mother and a nursery all keep them?

Marking rubric.

Criterion	Weight	Full marks
The formula	30%	Feeling named, limit held without argument, alternative offered; quiet and repeated
Tone and restraint	20%	No lecture, no bargain, no threat, no shaming
The rules map	25%	Every caregiver covered; contradictions named; the deciding question identified
Five rules	15%	Keepable by all caregivers; one is a safety rule; the child could understand each
Language advice	10%	Multilingual household supported rather than simplified; comprehension assessed first

The debrief question. "Which part of the formula did you drop when the child escalated?" Almost everyone drops the alternative, and the discussion of why — because it feels like a reward, because the parent is angry — is the week's most useful ten minutes.

4.6a When the household cannot agree

The case in 4.4 went well; most do not, and the failures are instructive.

The household that cannot agree because the disagreement is about something else. A couple who argue about whether their three-year-old may sleep in their bed. The rules map shows the disagreement is not about the child: the mother's own mother slept beside her until she was six, and the father's family regards a child in the marital bed as a boundary violation. The therapist who helps them agree about the child without naming the difference in their own histories will produce a rule that fails in a fortnight. The intervention is the history question, asked of each parent separately: "where did you learn what a family does at night?"

The household where the person with authority is not present. A father who works abroad and who telephones every evening with instructions. The mother's rules are superseded nightly, and the child has learned that the telephone is the real authority. The intervention is a single conversation with the father, scheduled, in which the five rules are agreed and his role is redefined: not the nightly auditor, but the endorser of the mother's authority, in one sentence said to the child. Fathers who agree to this change the household more than any other single intervention in this week's material.

The household where the caregiver is exhausted. A grandmother caring for two grandchildren under four while her daughter works, with no help and a bad knee. The five rules are agreed in ten minutes and abandoned by the second week, not because the household is uncoordinated but because the caregiver has nothing left. The intervention is not the rules; it is the relief — one afternoon a week, a nursery place, a neighbour — and the rules only work afterwards.

The household where the disagreement is being conducted through the child. Each parent has a rule, and the child is the site of the argument; the toddler's "he lets me" and "she is strict" is the household's conflict in a two-year-old's vocabulary. The intervention is the same as in the first case: the adults' disagreement is named, the child is removed from the argument, and the five rules are agreed as a truce rather than as a philosophy.

The common lesson. In every failure the technique was taught and the structure was not built. The formula and the five rules are instruments; they require a household that can agree, a caregiver who has capacity, and an authority structure in which the person doing the work has the standing to do it. A therapist who teaches the instrument without the structure has taught a technique the family will fail to use, and the family will conclude that they have failed.

4.7 Six errors in toddler work

1. **The advice avalanche.** Five techniques offered in one session, none of them practised. Fix: one formula, one rehearsal, one week.

2. **The child as the problem.** The assessment focused on the toddler's behaviour and the household left unexamined. Fix: the rules map, before any behavioural plan.
3. **The authority vacuum.** A plan that requires the parents to enforce rules they are not present to enforce. Fix: work through the caregiver who is there, and have the parents endorse that authority aloud.
4. **The missed hearing test.** A language delay assessed and managed without audiology. Fix: hearing first, every time.
5. **The subtractive language advice.** Telling a multilingual household to speak one language. Fix: each caregiver speaks their strongest language; increase sustained talk; assess comprehension.
6. **The shaming correction.** Any intervention that produces shame in the child or the caregiver. Fix: the limit without humiliation, applied to the adults too.

4.8 Culture, faith and the second year

Where the week's material travels well. The developmental facts about autonomy, language and limits are robust, and the clinical claim — that a household's coordination matters more than any single technique — is more obviously true outside the Western nuclear family than within it, because more households are multi-caregiver.

Where it strains, three ways.

The autonomy value. Erikson's stage names autonomy as the outcome, and many households name obedience. A child who is taught to comply first and choose later is not delayed; the family's curriculum is different. The therapist's task is to distinguish a curriculum from a problem: a household that teaches obedience and also allows the child to be a person is doing both, and a household that punishes every assertion is producing the shame the stage warns about. The distinction is behavioural, not cultural.

The physical punishment question. Many families use physical punishment, and the evidence on its effects is negative and consistent. The therapist's obligation is to state the evidence plainly, to be clear about the legal position where it applies, and to offer an alternative that the caregiver can actually use — which is the limit formula, taught with respect for the caregiver's intention. Moralising does not change behaviour; a workable alternative sometimes does.

The multi-caregiver household as the norm. The literature's advice assumes a parent who is present and has authority. In households where a grandmother, an aunt or a paid helper does the care, the advice must be addressed to them, and the parents' role must be defined rather than assumed. This is the week's central cultural point, and it is the reason the case is written the way it is.

Faith and the second year. Naming, dedication, first prayer, first fast, and the child's introduction to the community's rules are all part of the second and third years in religious households. The therapist should ask what the household expects of a two-year-old in religious terms, because the answer often contains a standard the parents are failing by and a resource they have not used — a grandparent, a community, a routine.

> **DEBATE**
> "Physical punishment is a cultural practice and a therapist should not impose a view about it." Argue both sides, and say what the therapist is professionally obliged to do.

4.9 Evidence note: robust development, contested practice

Well established. The sequence of language milestones, with wide ranges; the importance of comprehension as against production in the differential; the effectiveness of hearing assessment as the first step; the association between sustained, contingent talk and language development; the association between consistent, non-humiliating limit-setting and later self-regulation; and the negative association between physical punishment and child outcomes, which holds across cultural contexts in the research, though the effect sizes vary.

Contested. Whether the vocabulary count or the comprehension assessment is the better screen; the optimal amount of screen exposure in the second year, where the evidence is clearer about displacement of interaction than about the screens themselves; the effectiveness of parenting programmes delivered in groups versus individually; and the extent to which findings on physical punishment are confounded by the family contexts in which it occurs, which is the strongest version of the sceptical case and does not overturn the association.

What the popular literature gets wrong. That the "terrible twos" are a period to be survived rather than a developmental achievement; that a child who does not speak by two is "just lazy"; that bilingual households should simplify; and that consistency means rigidity. Each is common in the advice families receive, and each requires correction.

What the practitioner may claim. "The second year is the beginning of the self, and the household's task is to hold limits without humiliating the child; language delay is assessed by hearing, comprehension and play, not by word counts; and the intervention that works is an agreement between the adults who actually provide the care." Each claim is defensible.

4.4a The same household, six months later: what changed and what did not

A follow-up section, because the course's cases should show the ordinary unevenness of clinical change.

What changed. The shouting at the grandmother reduced from "several times a day" to "maybe twice a week". The five rules were in use: the grandmother could recite them, the mother could, and Kola could recite three of them. Tunde's language was at age level in Yoruba. The grandmother and the mother had stopped contradicting each other in front of the child, mostly.

What did not change. The parents were still absent during the week. The uncle still gave sweets, which nobody had included in the five rules, and which produced one predictable argument every Sunday. The grandmother still believed that the mother was

"too soft", and said so to the therapist, once, when the mother was not in the room. And the household's crowding had worsened, with a cousin moving in.

What the therapist did with the things that did not change. Three decisions, each recorded. The uncle's sweets were left alone: they were not a safety issue, and the household had chosen five rules that did not include them, which is what "everything else is each person's own business" means in practice. The grandmother's view of the mother was noted in supervision as a likely source of future difficulty, and not raised with her, because raising it would have required the therapist to take a side. And the cousin's arrival was added to the map, with one question to the grandmother: "what will be different for Tunde now that there is one more person in the room?"

Why this section exists. Students reading case studies learn that therapy produces change in the areas it addresses and leaves the rest, and that a therapist who tries to change everything produces a family who comply in the room and revert at home. The discipline is to choose the smallest change that matters, get it into the household's routine, and leave the rest to the household.

4.4b Two toddlers, two very different assessments

A comparison, to make the differential concrete.

Toddler one. Twenty-eight months. Twenty words. Understands everything: fetches named objects, follows two-step instructions, points to pictures on request. Uses gesture and vocalisation richly. Plays symbolically, feeding a doll. Hearing untested; family history of late talking (his father spoke at three).

Assessment: expressive language delay, with good comprehension and symbolic play. Action: hearing test (normal); language stimulation advice; speech and language therapy referral; review in three months. Prognosis: good, and the family should be told so plainly.

Toddler two. Twenty-eight months. Twenty words. Does not follow instructions unless they are accompanied by a gesture; does not point to show, only to obtain; does not bring objects to share; plays repetitively with one toy; distressed by a change of route to the nursery.

Assessment: the language delay is accompanied by differences in social communication and flexibility, and the differential now includes a broader developmental assessment. Action: audiology, paediatric referral, and a speech and language assessment; the family work continues alongside, with attention to the parents' own distress and to the household's routines.

What distinguishes them. Not the word count — both have twenty — but comprehension, gesture, shared attention, symbolic play and flexibility. The comparison is the reason the week teaches an assessment sequence rather than a vocabulary target.

The conversation with the parents of toddler two. The hardest conversation in this week's material. What to do: name what is observed without naming a diagnosis; state what will be assessed and why; say what will not change, which is the parents' relationship with the child; give one thing to do this week that is useful whatever the

outcome (a daily ten minutes of following the child's lead); and set the next appointment before they leave. What not to do: reassure falsely ("he will catch up"), speculate about causes, or deliver a diagnosis in a corridor.

4.5a The language history, asked as a conversation

Six questions, with what each produces, because the history is where the differential is decided.

One — "who speaks to him, in what language, and how much of the day?" Produces: the exposure map, and the frequent finding that the child hears commands in three languages and conversation in none.

Two — "when he makes a sound, what happens?" Produces: the contingency of the response. A household that does not answer the child's sounds is not depriving him; it is not yet in conversation with him, and that is changeable in a week.

Three — "what does he understand that he does not say?" Produces: the comprehension-production gap, which is the single most useful piece of information in the differential.

Four — "does he point, and does he look at you when he points?" Produces: shared attention, the marker that distinguishes a language delay from a broader developmental difference.

Five — "is he different at nursery?" Produces: the setting comparison, which identifies the child who is silent in one place and fluent in another.

Six — "what did you notice first, and when?" Produces: the onset and the parents' own chronology, which is often more accurate than the referral.

How the six are used. In a single conversation of fifteen minutes, in the family's language, with the child present so that the answers can be checked against what the therapist sees. The written summary goes in the record with the hearing status, the comprehension judgement, and the referral decision — because those three are what the next clinician will need.

4.10 Key terms

Term	Definition
Autonomy versus shame and doubt	Erikson's second stage: the self that can act, or the self that is judged
Developmental function of "no"	The word establishes separateness and the first experience of agency
Language explosion	The growth from about fifty words to three hundred and to two-word combinations in the second year
Comprehension–production gap	Understanding more than one can say; the key variable in the language differential
Late talker	Normal variation with good comprehension and often a family history
Shared attention	Following and directing another's attention to an object; a marker distinguishing types of delay

Term	Definition
Symbolic play	Using one object for another; the foundation of imagination and narrative
Limit formula	Name the feeling, hold the limit, offer the alternative
Rules map	A drawing of every caregiver's rules and the points of contradiction
Five rules	A deliberately small agreed set that a multi-caregiver household can keep
Subtractive advice	Telling a multilingual household to abandon a language; not supported by evidence
Coordination	The household's agreement about who decides and what the rules are

4.11 Self-test

1. What is the developmental function of the word "no", and what happens in a household that cannot tolerate it?
2. Give the three parts of the limit formula and explain why the order matters.
3. Name the five-way differential for language delay and the assessment that decides it.
4. Why is comprehension more useful than production in the assessment?
5. What is the "rules map" and what does it reveal?
6. Why is the multi-caregiver household the commonest presentation in toddler work?
7. What does the evidence say about physical punishment, and what is the therapist obliged to do?
8. What distinguishes a language delay from a broader developmental difference?

Answers. 1. It establishes separateness and agency; a household that cannot tolerate it produces either a compliant child or an escalating one, and both present in clinics. 2. Name the feeling, hold the limit, offer the alternative; the order matters because a distressed child cannot process a limit until understood, a limit without the feeling teaches that feelings are inadmissible, and an alternative gives the refusal somewhere to go. 3. Late talker; multilingual; hearing loss; global delay; autism spectrum presentation — decided by audiology, comprehension assessment, play observation and the language history. 4. Because it separates a child who cannot produce from a child who cannot understand, which are different clinical situations and different prognoses. 5. A drawing of every caregiver's rules and their contradictions; it reveals that "he does not listen" is usually a household's uncoordinated rules. 6. Because most households with toddlers have more than two caregivers, and the literature's advice assumes a parent present with authority. 7. The association with negative outcomes is consistent across contexts; the therapist is obliged to state the evidence, be clear about the legal position, and offer a workable alternative rather than moralise. 8. Shared attention, gesture, symbolic play and flexibility — not the vocabulary count.

4.12 Reflective task

Write down five rules that were in force in the household where you grew up. Then write the five rules you would want in a household with a two-year-old. Compare them, and write three sentences on what the comparison tells you about your own developmental curriculum — the assumptions you will bring into a family's living room without noticing. Bring it to the seminar. The exercise is the week's cultural material applied to the practitioner, and most students discover at least one rule they have been enforcing in other people's houses without knowing where it came from.

4.13 Readings for this week

- **Erikson (1963)**, Childhood and Society, the second stage, read alongside the critique of the stages as culturally located.
- **Lieven (1994)** or **Tomasello (2003)**, Constructing a Language, for the language development this week relies on.
- **Rescorla (2011)**, on late talkers, for the evidence about prognosis and the case for early referral.
- **Webster-Stratton (2005)**, on parenting programmes for young children, for the evidence behind the limit formula.
- **Gershoff (2002, 2016)**, the meta-analyses on physical punishment, read carefully, including the critique.
- **Rogoff (2003)**, again, for the multi-caregiver household.

Read Rogoff before the parenting literature; the corrective first. For the case analysis, take the language history of a real child (anonymised) and defend your differential in three sentences. The reading-log question this week is: "whose rules was I raised by, and how do they appear in my clinical advice?"

Next: Week 5 — Early childhood: play, theory of mind, initiative

---

Week 5 — Early childhood: play, theory of mind, initiative

Developmental tasks. Initiative versus guilt (Erikson, roughly three to six); the flowering of pretend play; theory of mind (the understanding that another person can hold a belief that is different from the truth, typically consolidating between four and five); moral reasoning that begins as consequence-based and slowly becomes intention-based; and school readiness, which is less about letters than about the capacity to hold a plan, wait, and recover from frustration.

Play is the work. Play develops language, self-regulation, perspective-taking, and the capacity to tolerate frustration. It is also the child's first research method: a child who pretends is running experiments on what other minds contain. Children who have experienced adversity often play less symbolically and more repetitively — a clinical observation that underpins play therapy (MFT 727) and that a family therapist must be able to read, because the playroom is often the first place a four-year-old tells the truth.

The clinical skill: observing play without hijacking it. Describe what the child does; do not direct it, and do not interpret it prematurely ("that must be your father"). Interpretation before trust produces a performance, not a communication. The therapist who says "I wonder if the lion is angry because someone left" has stopped watching and started theorising, and the child will now either agree with the adult or leave.

Why this week sits in a family therapy course. The referring complaint is almost never about play. It is "he is aggressive at school", "she will not separate from her mother", "he does not listen to anyone", "she has started lying". Each of those is a developmental question wearing a behavioural costume, and the assessment that answers it is the same one: what can this child do in play, and who is in the room when she does it?

> **CASE**
> A mother brings her four-year-old because the nursery reports that she "plays the same thing every day" and "does not join the others". The mother has recently returned to full-time work after a year at home, and the child's grandmother has moved in. Assessment finds: the child's play is rich but narrow, restricted to one script about a mother leaving and returning; her theory-of-mind performance is age-appropriate; her language is above age; and she separates from her mother without distress but does not greet her on reunion. The problem is not the child's development. It is the family's story about the separation, which the child has been asked to carry alone.

5.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The fourth year	What changes between three and six, and why the presenting complaint is never about play	List the complaints that brought children to their last placement
10–22	Initiative versus guilt	Erikson's pair; the difference between a child who starts things and one who asks permission	Rewrite four adult sentences that produce guilt
22–38	Play as development	The six levels of play, from sensorimotor to cooperative drama	Code ninety seconds of video against the six levels
38–52	Theory of mind	The false-belief task, demonstrated live, with a volunteer	Predict a four-year-old's answer and explain the error
52–64	Moral reasoning	Consequence-based to intention-based; the broken-cup stories	Interview a child about a small transgression and code the reasoning
64–76	Observing without hijacking	The four skills, demonstrated with a puppet, then with a volunteer	Practise ten minutes of child-led play; count their own questions
76–86	Reading the play	Repetition, constriction, and the differential between adversity, autism, and ordinary development	Write one paragraph on what the play does not show
86–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Play is the part of this week that students most want to do, and the part they most often do badly, because doing it well means doing very little. The lecture therefore has to make boredom respectable. A therapist who can sit through ten minutes of a child putting the same bear in the same box and say only "the bear is in the box" has acquired a clinical skill that takes longer to learn than any technique in this book. The demonstration should include the lecturer failing — asking a question, making a suggestion — and then naming the impulse that produced it.

The sentence the lecture must land. "A child's play is not a message to be decoded by the adult; it is a conversation the child is having with herself, and your job is to be a good listener rather than a clever one."

5.2 Five concepts that organise the week

1. Initiative versus guilt. Erikson's third stage. The child now has a body that can do things, a language that can propose things, and a will that can insist on things. The developmental gain is the capacity to start — a game, a plan, a joke, an argument. The developmental risk is guilt: the conviction that wanting to start something is itself a fault. The family's contribution is decisive and it is made in small sentences. "Not now" repeated a hundred times produces a child who stops proposing. "Tell me your plan and we will see" produces a child who proposes.

The clinical translation is precise: when a five-year-old cannot initiate, the assessment is not only of the child but of the last two years of responses to initiation. Ask the parents what happens when the child asks for something. The answer is usually a rule about the household's exhaustion rather than about the child.

2. Pretend play as the engine of self-regulation. In pretend play the child holds two things in mind at once — the real situation and the imagined one — and this dual representation is the same capacity that later allows a child to hold a rule in mind while being provoked. Vygotsky's observation that play is "the leading source of development" rests on this. The implication for assessment is that the quality of a child's pretend play predicts school adjustment better than the child's vocabulary.

3. Theory of mind. Between roughly three and five the child moves from a "copy" theory of mind (everyone sees what I see, knows what I know) to a representational one (people act on their beliefs, and beliefs can be false). This is measured by the false-belief task, and it is the developmental foundation of every clinical skill the child will ever need: empathy, deception, tact, negotiation, and the understanding that a parent can be angry and loving at once.

4. Moral reasoning begins as consequence-based. Piaget's paired stories: a child who breaks one cup while helping versus a child who breaks fifteen while stealing jam. Children under about seven judge the first child as naughtier, because the outcome is larger. They are not being immoral; they are reasoning with the only tool they have. The clinical consequence: do not ask a five-year-old "was that kind?", ask "what happened next?" The first question is beyond the stage; the second builds the stage.

5. School readiness is regulation, not letters. The strongest predictors of a good first year are the capacity to follow a two-step instruction, to wait, to recover from a small frustration, and to seek help from an adult. A child who arrives at school able to do those four things will learn to read. A child who arrives knowing the alphabet but not able to wait will not. For the family therapist this reframes the "school readiness" conversation from coaching to structure: bedtime, breakfast, a predictable goodbye, and an adult who is reliably there at the gate.

5.2a Theory of mind: five tasks and what failure means clinically

Theory of mind is not a single achievement. It is a set of understandings that appear in a rough order, and knowing which one a child has tells the therapist what kind of conversation is possible with her.

Task 1 — Desire (18–24 months). Two bowls, one with biscuits, one with broccoli. The adult says she wants the broccoli and asks for the bowl. The child who hands over the broccoli has understood that the adult's desire differs from her own. Failure here is not yet diagnostic of anything except age.

Task 2 — Visual perspective (2–3 years). A card with a cat on one side and a dog on the other, held between two people. "What do you see?" "What do I see?" The child who answers "a cat" to both has not yet grasped that seeing is situated. This is the basis of the clinician's later ability to notice that two family members are describing the same event from different positions.

Task 3 — False belief (4–5 years). The classic: a doll puts chocolate in a cupboard and leaves; a second doll moves it. "Where will the first doll look?" The child who says "in the new place" has answered from her own knowledge. This is the task that most reliably indexes the shift.

Task 4 — Appearance versus reality (4–5 years). A sponge painted to look like a rock. "What does it look like? What is it really?" The child who cannot hold both at once will also struggle with the clinical insight that a family looks one way and is another.

Task 5 — Second-order belief (6–8 years). "She thinks that he thinks…" This is the machinery of social strategy, and its arrival is why middle childhood is the age of secrets, clubs, and exclusion.

What failure means, clinically. A child who fails the false-belief task at four is unremarkable. The same child at six, with good language, is a reason to look further — and the look is a developmental assessment, not a diagnosis from the therapy room. Three cautions.

- **Language.** Every one of these tasks is delivered in language. A child with a language delay will fail tasks she may understand. Test in the child's stronger language, and if the therapist cannot, bring an interpreter rather than a simplified script.
- **Culture.** The tasks assume a particular conversational convention — that an adult asking a question to which they know the answer is testing, not conversing. Children who have been taught that it is rude to contradict an adult may answer "the cupboard" because the adult is looking at it.

- **Trauma and vigilance.** A child who has had to track a volatile adult's mood may outperform on second-order tasks and underperform on first-order ones, because vigilance trains attention to intention rather than to belief. This is not a deficit; it is an adaptation that costs something elsewhere.

The clinical use of the tasks. Not to score the child, but to calibrate the session. A four-year-old who fails false belief cannot be asked to "think how your mother felt". She can be asked to "show me with the dolls". The therapist who knows where the child is on this ladder stops asking questions the child cannot answer, which is the commonest way well-meaning adults make small children feel stupid.

5.2b Six levels of play and what each one tells you

Play has a developmental ladder, and each rung is a piece of assessment data. The ladder below is adapted from the classic social-participation scheme, with the clinical read added, because the read is what a therapist needs.

Level	What it looks like	Age it is typical	What it tells the clinician
1. Sensorimotor / practice	Repetitive movement: banging, filling, pouring, spinning	0–18 months	Still present at five only if development is disrupted or the child is regulating
2. Solitary	Playing alone, absorbed, not attending to others	2–3 years	At six, in a busy room, this is information about the child's state, not her sociability
3. Onlooker	Watching others play, asking questions, not joining	2–4 years	At six, suggests the child has been excluded or has learned that joining is unsafe
4. Parallel	Playing beside another child with the same materials, without exchange	3–4 years	A stable level at six suggests the child cannot yet negotiate
5. Associative	Sharing materials, borrowing, no shared goal	4–5 years	The level at which most nursery complaints are actually generated
6. Cooperative / dramatic	Shared goal, assigned roles, rules, a story that survives conflict	5–7 years	The best single predictor of school adjustment in this age band

Reading the ladder rather than a score. The question is never "what level is this child at?" but "what does this child do when the level is too hard?" A child who retreats to level 1 under stress is regulating. A child who attacks is also regulating, badly. A child who goes blank has stopped regulating, and that is the observation that should change the therapist's next move.

Three patterns that matter clinically.

Constriction. The child plays, richly, but inside one script. The script is repeated with minor variations, and any attempt to introduce a new element is refused. Constriction is common after a disruption the child could not narrate — a separation, a death, a move, a new baby, a hospital stay. The script is not a symptom to be removed; it is the child's only available formulation, and the clinical task is to help it become more flexible rather than to replace it.

Repetition without elaboration. The same act, many times, with no development across sessions. This is the pattern that most often indicates adversity, and it is also the pattern of autistic play when the repetition is sensory rather than narrative. The differential is not made in the playroom alone: it is made with a developmental history, an observation of communication, and a proper assessment.

Play that stops at the door. The child plays fluently at nursery and cannot play at home, or the reverse. This is almost always a fact about the household's state rather than the child's capacity, and it is the observation that turns a child referral into a family one.

What to record. Five lines, in this order: what the child chose; whether the play had a story; how the child responded to a new element; what happened when the play was interrupted; and how the child ended. Those five lines are more useful than a page of adjectives, and they can be compared across sessions by two different therapists.

5.3 Observing play without hijacking it

The method is four skills and one prohibition. It is taught here, in the second year of life course, because it is the foundation of every later child-facing technique and because it is the hardest thing in this book to do for ten consecutive minutes.

Skill 1 — Track. Sit at the child's level, at a slight angle rather than opposite, and let your eyes follow the child's hands rather than her face. Tracking communicates the thing that cannot be said: I am interested in what you are doing, not in what you will produce for me.

Skill 2 — Describe. Say what the child is doing, in the present tense, without evaluation. "You are putting the small one inside the big one." Not "good job", not "what a clever tower", not "is that a house?" Description gives the child a narrator, and the narrator's vocabulary becomes the child's.

Skill 3 — Reflect. When the child speaks, say it back, slightly expanded. Child: "Bear fall." Adult: "The bear fell down and he is stuck." This is the single most efficient language intervention available to a non-specialist, and it is also the most efficient way to show a child that she has been heard.

Skill 4 — Imitate. Copy one thing the child does, exactly, and wait. Imitation is a turn-taking offer that does not require words, and for a child who finds adults unpredictable it is the least threatening thing an adult can do.

The prohibition — do not interpret. No "I wonder if…", no "that must be your daddy", no "you seem angry". There are three reasons. The first is epistemic: at this age the therapist does not know, and a confident wrong interpretation teaches the child that adults invent meanings. The second is interactional: an interpretation recruits the child into confirming the adult's theory, which converts play into a performance. The third is developmental: interpretation requires exactly the representational capacity the child is still acquiring.

The one exception. If the child's play depicts something dangerous and the child addresses the therapist directly — "the man is coming" — the therapist may reflect the

child's own words and stay alongside: "The man is coming and you are holding the door." That is still not interpretation. It is company.

> **PRACTICAL**
> Ten minutes of child-led play with a volunteer child or a peer. Your only tools: describe, reflect, imitate, and set one limit. A partner times you and tallies your questions, suggestions, and praise. Aim for fewer than three questions, fewer than two suggestions, and — the hardest target — no praise at all. Debrief on the impulse behind each one.

Why "no praise" is in the exercise. Praise evaluates, and evaluation moves the child's attention from the activity to the adult's face. A child who is praised for a drawing learns to check whether the drawing is acceptable. A child who is described learns to look at the drawing. The exercise is not a claim that praise is harmful; it is a demonstration of how much of an adult's ordinary talk is evaluation, and how quickly a child adapts to it.

5.3a Setting a limit in the playroom

Sooner or later the child will do something that cannot continue: hit, throw at the window, climb the shelf, hurt herself. The limit is part of the method, not a failure of it, and it is set in four moves.

Move 1 — Stop the action, not the child. A hand on the toy, not on the child. "I am going to hold the block."

Move 2 — Name the limit as a rule of the room. "In this room we do not throw blocks at people." The rule belongs to the room, not to the therapist's mood, and it will be the same next week.

Move 3 — Name the feeling and offer the alternative in one breath. "You wanted me to look and I was writing. You can throw the block at the floor, or you can say 'look'." Two options, both acceptable, one of them the one you want.

Move 4 — Stay. Do not leave, do not lecture, do not withdraw warmth. The child's learning is that the relationship survives the limit. This is the same lesson as Week 4's limit formula, at a different age and with a different vocabulary, and the continuity is the point: the family that learned "you may be angry" at two is being asked to say the same thing at four.

What not to do. Do not ask "why did you do that?" — a four-year-old cannot answer and will invent. Do not remove the toy as punishment unless the rule requires it. Do not say "I am not angry" if you are, because children read faces more accurately than words at this age, and the mismatch teaches them to distrust their reading.

5.4 Three procedures for the clinic

Procedure 1 — The five-minute play observation, with the parents watching. Before any history, ask the parents to sit at the side of the room and watch the therapist play with the child for five minutes. Then ask one question: "what did you notice?" The parents' answer is the assessment. Some will describe the child's cleverness; some will describe the child's problems; some will describe nothing and ask what they were supposed to see. Each answer tells the therapist where the family's attention lives, and the next move is to give them one thing to look for next time.

Procedure 2 — The false-belief demonstration, used as a conversation. With a child of four or five, use two dolls and a small box in front of the parents. Run the task once. Then say to the parents, in front of the child and without scoring it, "she answered from what she knows; most children her age do, and it changes in the next year or so". The purpose is not diagnosis. It is to give the parents a developmental fact that reframes a behaviour they have been treating as defiance — usually the child's insistence that her account of an event is the only account.

Procedure 3 — The interrupted-play observation. Ask the parent to play with the child for five minutes, then to stop and say "we have to go now". Watch what happens in the next ninety seconds. Three things are being observed: whether the child can use the parent's face to regulate, whether the parent offers a bridge ("one more turn, then shoes"), and how quickly the pair recover. This ninety-second window is more informative than an hour of interview, and it takes no equipment.

5.5 The first session with a four-year-old

Structure the hour so that the child is present for the part that concerns her and absent for the part that does not.

Phase 1 — The room (5 minutes). Child and parents enter together. Name the rules of the room in the child's language: "we stay in this room, we do not hurt, and you choose what we play". The rules are for the child; the parents are also listening, and they are learning what a rule sounds like when it is not a threat.

Phase 2 — Play with the child, parents watching (15 minutes). Therapist plays, using the four skills. Parents sit where the child can see them. This is simultaneously assessment, demonstration, and — for many parents — the first time they have watched an adult be interested in their child without correcting her.

Phase 3 — The child leaves (5 minutes). Do not sneak her out. Say what is happening: "you are going to wait with Auntie Bisi while I talk to your mum and dad." A child who is told what is happening learns that adults can be trusted with information.

Phase 4 — Debrief with the parents (25 minutes). Three questions, in order. What did you notice? What does this look like at home? What have you already tried? Then one piece of developmental information, given plainly, that bears on the referral.

Phase 5 — One task (10 minutes). Not a programme. One thing: usually ten minutes of child-led play a day, or one change to the goodbye, or one sentence to use instead of the sentence they are using. Write it down. Ask them to notice what happens, not to succeed.

Why the order cannot be reversed. If the parents are interviewed first, they arrive at the play phase having already told the therapist what is wrong, and the therapist will see it. The five minutes of uncontaminated observation is the most valuable data in the session, and it is lost the moment the therapist has a theory.

5.6 Case: the child who plays the same thing every day

Referral. Zainab, four years and two months, brought by her mother, Nkechi, 33, a bank officer, on the recommendation of the nursery. The nursery's written note says: "Zainab plays the same game every day. She does not join group activities. She is very bright but very alone."

Household. Mother Nkechi, father Emeka, 37, an engineer who travels two weeks in four. Paternal grandmother, 61, moved in eight months ago after a fall. A baby brother, six months old. A house help, 19, who does the school run. Languages: Igbo at home, English at nursery; Zainab's English is fluent and her Igbo comprehension is good.

History. Pregnancy and birth unremarkable; milestones on time; no illnesses; hearing tested at three, normal. The change the mother can date precisely: eight months ago, when the grandmother moved in and the mother returned to work within the same month. "Before that she was the one who talked to everybody. Now she talks to nobody."

The play observation. Zainab chooses the doll's house. She puts the mother doll in the car and drives it to the edge of the rug. She says, quietly, "bye". She waits. She drives back. She says, "I'm home". She does this eleven times in ten minutes, with the same two sentences, at the same pace. The therapist describes, reflects, imitates the driving once. On the twelfth repetition the therapist drives her own small car to the other side of the rug and parks it. Zainab looks at it, does not touch it, and continues.

At minute nine the therapist says only: "the mama keeps coming back." Zainab stops, looks at the therapist for perhaps two seconds, and says: "she always comes back." Then she resumes.

What the play shows. A child with intact symbolic capacity, a coherent narrative, age-appropriate language, and a single theme: departure and return. The theme is not a symptom. It is a formulation — the child's working model of the thing that changed, expressed in the only medium available to her. The constriction is the finding: eleven repetitions, no elaboration, no new element tolerated.

What the play does not show. It does not show autism. Zainab's communication is reciprocal, her language is above age, her play is narrative rather than sensory, and her eye contact — brief, but present and socially timed — is not what autism looks like. It does not show attachment disorder. She separated without distress and greeted her mother on reunion, which is the pattern of a secure child managing a hard change. It does not show trauma in the sense of a single event; it shows a disruption of arrangement, which is a different clinical object.

The differential, written out. Four candidates, and the evidence for and against each.

Candidate	For	Against	Verdict
Normal adjustment to change	Dated onset; coherent theme; good development otherwise	Eight months is long for uncomplicated adjustment; no elaboration at all	Plausible but incomplete
Autism spectrum	Repetitive play; social isolation at nursery	Reciprocal communication; fluent imaginative language; narrative (not sensory) repetition	Not supported
Attachment difficulty	Reunion greeting is muted; theme of departure	No indiscriminate behaviour; no freezing; separates comfortably	Not supported as a disorder
Family-system disruption	Onset coincides with return to work and a new adult in the house and a new baby; father absent half of each month	—	Primary formulation

The formulation. Zainab is not ill and does not need a child therapy referral. She has been asked, at four, to hold a change that nobody explained to her, in a household where the adults themselves have not agreed what the change means. Her play is the most articulate statement of the family's situation in the room. The intervention is with the adults, plus a small, precise piece of work with Zainab about goodbyes.

The session with the parents. The therapist shows them nothing of the play — she does not narrate it back as evidence, which would make the child's private material into an exhibit. Instead she asks three questions.

1. "Who told Zainab that you were going back to work?" (Answer: nobody; it was decided in a conversation the child was not in.)
2. "What happens at the door in the morning?" (Answer: the house help takes her; the mother leaves before she wakes "because it is easier".)
3. "When she says 'Mummy, don't go', what do you say?" (Answer: "I have to work, so you can have things." Followed, the mother admits, by leaving the room.)

The intervention, in four parts.

Part 1 — A goodbye with a shape. The mother wakes the child fifteen minutes earlier and does the handover herself. The goodbye has four elements, always in the same order: a warning ("in two minutes I am going"), a physical routine (a hug, then a handover to the grandmother, not to the house help), a sentence ("I always come back, and I will be here before your bath"), and a departure that is not hurried. The mother's objection is predictable and worth answering: "she will cry." She will. The alternative is a child who learns that departures happen without warning, which is exactly what she has learned.

Part 2 — The return is the intervention. The evidence that matters to a four-year-old is not the promise but the arrival. The mother commits to being home before the bath three evenings a week for a month, and to the first five minutes after arriving being with Zainab and nobody else — no phone, no report from the grandmother, no baby in her arms.

Part 3 — The grandmother's authority, said aloud. The therapist asks the mother and the grandmother to agree, in the session, on the two things the grandmother decides

without appeal (bedtime, and safety) and the two things the mother decides without appeal (school, and what Zainab is allowed to watch). Then each says to the child, in front of the other, "Grandmother is in charge of bedtime, and I am in charge of school." The sentence is for the adults' benefit as much as the child's: a four-year-old who is caught between two authorities will stop proposing anything, which is the definition of guilt in Erikson's terms.

Part 4 — A play prescription, of a peculiar kind. The mother is asked to play the doll's house game with Zainab, once, for ten minutes, and to let Zainab direct it. She is not asked to add anything. She is asked to notice what her daughter does with the mother doll, and to report one observation next session. This is not play therapy; it is the mother being given access to her child's account of the change without anyone having to interpret it.

5.6a Six weeks later

The mother reports three changes, in her own order: Zainab now cries at the door in the morning, which the mother finds harder and the therapist regards as progress, because a child who protests a departure has decided that the departure is worth protesting; she greets her mother at the door in the evening and takes her hand; and the doll's house game has acquired a new element — a grandmother doll, which Zainab puts in the kitchen and moves, once, to the front door.

The nursery reports she has joined a group of three girls in a game with assigned roles. The repetition has not disappeared. It has become a game the child plays with other children, which is what elaboration looks like at four.

The father, who was on a trip for the first three sessions, attends the fourth by telephone and the fifth in person. He says very little and cries once. The therapist does not interpret the crying. She asks him what he would like his daughter to know about why he travels, and he says, "that I am coming back." The therapist suggests he says it to Zainab, at the door, every time he leaves, and he does.

What would have changed the plan. Three findings would have redirected this case, and the student should be able to name them. If Zainab had failed language comprehension tasks in Igbo as well as English, the referral would have gone to audiology and speech therapy before any family work. If her play had been sensory rather than narrative, with no social referencing, the referral would have gone for a developmental assessment. If the reunion greeting had been absent altogether, with freezing or indiscriminate approach to the house help, the formulation would have shifted toward caregiving disruption and the safeguarding question would have been asked explicitly.

5.7 Transcript: ten minutes of play, annotated

The transcript below is the observation described above, with the therapist's internal reasoning annotated. Read the annotations as the discipline the exercise is trying to install: every line of therapist speech is classified by which of the four skills it uses, and every impulse that was resisted is named.

> **T1**
> (Zainab puts the mother doll in the car; says nothing.) **Therapist:** "The mama is getting into the car." — Description. No question. Note the temptation: "where is she going?" would have converted the child's play into an answer for the adult. **T2** (Zainab drives the car to the edge of the rug.) **Therapist:** "She is driving away." — Description, present tense, short. **T3** (Zainab, quietly: "bye".) **Therapist:** "Bye." — Reflection, at the same volume. Matching volume is a skill; a therapist who says "BYE!" has taken the play away from the child. **T4** (Pause of about fifteen seconds. Zainab does not move.) **Therapist:** (says nothing) — The hardest line in the transcript. The impulse to fill the silence is the therapist's, not the child's. **T5** (Zainab drives back. "I'm home.") **Therapist:** "She came home." — Reflection with a grammatical expansion. The child hears her own sentence completed. **T6** (Repetition three.) **Therapist:** "She went, and she came home." — Description that holds the whole cycle in one sentence, which the child has not yet done aloud. **T7** (Repetition seven. The therapist's impulse: "does she always come home?" — resisted.) — A question would have asked the child to state a belief about the future, which is precisely the capacity under construction. Do not test what is being built. **T8** (Repetition ten. The therapist drives a second car to the other side of the rug and parks it.) — A non-verbal offer. It changes the scene without requiring the child to accept a suggestion and without claiming a role. If she ignores it, nothing is lost. **T9** (Zainab looks at the second car, does not touch it, continues.) **Therapist:** (nothing) — The child declined. The offer was made correctly and its refusal cost nothing. **T10** (Minute nine. "The mama keeps coming back.") — The only interpretive-adjacent statement in the session, and it is a description of the play's pattern, not of the child's feelings. It contains no "I wonder", no "you feel", and no named person from the child's family. **T11** (Zainab stops. Looks at the therapist. "She always comes back.") — The child has made a statement about the world. This is the moment the therapist has been working toward for nine minutes, and the correct response is the smallest possible one. **T12 Therapist:** "She always comes back." — Reflection. Nothing added. No "and mummy always comes back too." The child made a general statement; the therapist does not force it to be about her mother. If the child wants to make that link, she will, in her own time, and probably not in this session.

5.8 Laboratory: the ten-minute observation, marked

Aim. To produce a written observation of a child's play that another clinician could use, and to demonstrate command of the four skills under supervision.

Procedure. In pairs, with a volunteer child (or, where a child is not available, with a peer instructed to play at the level of a four-year-old — a surprisingly demanding role). One student plays for ten minutes using only the four skills and one limit. The second student times the play, tallies questions, suggestions, praise, and interpretations, and writes the five-line record. Roles swap. Then each pair writes a single paragraph answering: "what did this child's play tell me that an interview would not have?"

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Restraint	More than six questions or three suggestions	Three to six questions, one or two suggestions	Fewer than three questions, no suggestions, no praise
Description	Evaluative or absent	Present but intermittent, with judgement words	Continuous, present tense, non-evaluative, matched to the child's attention
Reading the play	Describes the child's personality	Describes actions only	Names level, theme, elaboration, and response to a new element
The limit	Not set, or set with threat or withdrawal	Set, but with explanation and a lecture	Set in four moves, relationship intact, play resumed
The record	Adjectives and impressions	Actions without structure	Five lines in the required order, usable by a second clinician

The distinction that carries most of the marks. The difference between "she was anxious and withdrawn" (an inference with no evidence) and "she played the same two-sentence script eleven times and did not touch the second car" (an observation). Students reliably believe the first is more clinically sophisticated. It is the second that can be checked, taught, and compared.

5.9 Six errors

Error 1 — Interpreting before the child has told you anything. "I wonder if the lion is your father." The child either agrees, which teaches her that adults supply meanings, or stops playing. The error is not that the interpretation is wrong. It is that it is a question disguised as an observation and it recruits the child into the adult's theory.

Error 2 — Turning play into a lesson. Asking the colour of the block, counting the blocks, naming the animal. This is not play and it is not therapy; it is a test administered by someone who has run out of things to say. The child's developmental level is not improved by being quizzed at it.

Error 3 — Using praise as the default response. "Good job!" every thirty seconds trains the child to look at the adult after each action. The child who is watched learns to play; the child who is praised learns to perform. The distinction matters most for children whose households already require performance.

Error 4 — Reading repetition as pathology. Repetition is the mechanism by which this age group consolidates. The question is not whether the play repeats but whether it elaborates across sessions. A therapist who treats the eleventh repetition as a symptom will miss the twelfth, in which the child adds a grandmother doll.

Error 5 — Doing the child work and calling it family work. A weekly play session that never touches the household will produce a calm child who returns to the same arrangement. This is the characteristic error of clinicians trained only in child modalities, and it is why this course teaches play observation inside a family therapy programme: the observation is the door, and the family is the room.

Error 6 — Diagnosing in the playroom. Autism, attachment disorder, ADHD, and "trauma" are not diagnosable from a ten-minute observation, and a therapist who says

"this looks like autism" to a parent has made a claim that will govern the family's next two years. Describe what you saw, name what you cannot tell, and refer for the assessment that can.

5.10 Culture, faith and the child who plays

Play is not culturally neutral. The materials in a therapy room — a doll's house, a doctor's kit, a family of small figures — encode a household. A child who has never seen a doll's house will not necessarily be less competent; she will be less fluent with the props. The assessment must therefore include an observation with materials the child knows. In the Nigerian clinic the most useful additions are usually a market stall, cooking pots, a bus, and a church or mosque set, because those are the scripts most children have.

Who plays with whom. In many households the answer is "the other children", and a four-year-old who plays fluently with cousins and neighbours but not with adults is not delayed. The clinical question is whether the child has any setting in which she plays richly. If she does, the household has a resource the therapist can build on rather than a deficit to correct.

Faith and the child's explanations. By five, children in observant households have absorbed explanatory frameworks — prayer, protection, spirits, the will of God — and will use them in play. A child who stages a deliverance scene is not necessarily expressing distress; she may be rehearsing something her household does weekly. Ask the parents what she has been part of before deciding what her play means.

Discipline and initiative. This is the week's sharpest cultural tension. Erikson's "initiative" is cultivated in households that reward a child's proposals and tolerated in households that treat a child's proposal as insubordination. A therapist who measures initiative against a single cultural standard will misread a well-regulated child as inhibited, and will recommend a change the parents experience as an attack on their authority. The clinical question is not "does this child initiate?" but "in what domains is this child permitted to initiate, and what happens to her when she does?"

> **DEBATE**
> "The child's play belongs to the child, and the therapist has no right to describe it to the parents." Argue for: the playroom is the one place a child's material is not reported, and showing it to parents converts it into evidence in a case the child did not consent to. Argue against: the parents are the intervention, a therapist who learns something decisive and withholds it is practising on the family, and a child's private play is not confidential in the way an adult's disclosure is. Then take a position, and state the rule you would follow: what, precisely, would you never say to a parent about a child's play, and why?

5.11 Evidence note

What the research supports. The association between the quality of pretend play in the preschool years and later social competence and school adjustment is well established in longitudinal work, with effect sizes that are modest but consistent after controlling for language and family income. The false-belief literature is one of the most replicated bodies of findings in developmental psychology, with the four-to-five transition robust across dozens of cultures — though the timing shifts with schooling, sibling number, and the language of testing.

What is weaker. The claim that play causes these gains is much harder to establish than the claim that it correlates with them. Interventions that add play opportunities produce smaller effects than the correlational literature would predict, and the most defensible reading is that play is a context in which existing capacities are consolidated and displayed rather than a training programme that installs them. The clinical implication is modest and useful: observe play because it shows you what the child has, not because ten minutes of it will make her have more.

What is not supported. The strong form of the play-therapy claim — that non-directive play sessions alone reliably resolve identified clinical disorders in preschoolers — is not supported by meta-analytic evidence, which finds small effects that shrink when the comparison is an active control. Non-directive play remains a valuable engagement method and a window on the child's representational world. It is not a substitute for changing the household, and this week's case is the argument.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Constriction in play at four, in a stable household, resolves in most children within a year. The same constriction, in a household that is itself unresolved, tends to persist and to generalise — which is the argument for treating the arrangement rather than the symptom.

5.12 Key terms

Initiative versus guilt — Erikson's third psychosocial stage; the capacity to start things, balanced against the conviction that wanting is a fault.

Pretend (symbolic) play — play in which an object or action stands for something else; the developmental engine of dual representation and self-regulation.

False-belief task — the standard test of whether a child understands that another person can act on a belief that is untrue.

Representational theory of mind — the understanding that people act on beliefs rather than on the world directly.

Constriction — play that is rich but confined to a single, repeated script, with new elements refused.

Elaboration — the developmental marker that matters: the addition of new elements, roles, or resolutions across sessions.

Scaffolding — adult support calibrated to the child's current level, withdrawn as competence grows.

Dual representation — holding the real and the imagined situation in mind at once; the mechanism by which play builds self-regulation.

School readiness — the capacity to follow instructions, wait, recover from frustration, and seek help; a stronger predictor of first-year success than early literacy.

Social referencing — the child's checking of a caregiver's face to decide how to feel about an ambiguous event; the observation that most reliably distinguishes security from its absence in the second and third years.

5.13 Self-test

1. A four-year-old answers "the cupboard" on a false-belief task even though she watched the chocolate being moved. What has she done, and what should the therapist not conclude?
2. Give three reasons why interpreting a child's play prematurely is a technical error and not merely a matter of taste.
3. A nursery reports that a child "will not join in". Name four observations that would distinguish exclusion, shyness, language difficulty, and a developmental difference.
4. What is the difference between repetition that is consolidating and repetition that is constricted?
5. Why does the exercise forbid praise, and what does praise do to a child's attention?
6. A parent says "he knows exactly what he is doing — he is manipulating me". What developmental information changes the conversation, and how would you say it?
7. In the case of Zainab, name the three findings that would have changed the formulation, and where each would have sent the family.
8. What is the strongest argument against telling parents what their child did in the playroom?

Answers. (1) She has answered from her own knowledge — she has not yet represented the other's belief as different from the truth, which is normal at four. The therapist should not conclude that she is delayed, dishonest, or unable to understand other people; the task indexes one narrow capacity, in one language, under one conversational convention. (2) It is epistemically unjustified at this age; it recruits the child into confirming the adult's theory, converting play into performance; and it demands the very representational capacity the child is still acquiring. (3) Whether she is invited and refused; whether she watches from the edge (onlooker) or does not attend at all; whether she plays richly in her own language with one other child; and whether her play is narrative or sensory and repetitive. (4) Consolidating repetition elaborates across sessions and tolerates a new element; constricted repetition repeats identically and refuses any addition. The test is across sessions, not within one. (5) Praise evaluates, and evaluation moves attention from the activity to the adult's judgement; a praised child checks the adult, a described child looks at her own work. (6) At four the capacity to hold another's belief in mind is still forming, so "manipulation" — which requires representing what the parent believes and

intending to change it — is usually not available; say that plainly, and replace the word with a description of what the child actually did. (7) Failed comprehension in the home language → audiology and speech therapy; sensory rather than narrative repetition with no social referencing → developmental assessment; absent or indiscriminate reunion behaviour → caregiving disruption and the safeguarding question. (8) The child cannot consent, cannot correct the record, and will live with the parents' new knowledge of her private material; the counter-argument is that the parents are the treatment.

5.14 Reflective task

Watch a child between three and six for ten minutes and write the five-line record. Then write, in one paragraph and without naming anyone, what you found yourself wanting to do and did not. Most students find the impulse is to teach, to test, or to be liked. Name yours. Then answer: which of the three would have been hardest for the child to refuse?

5.15 Further reading

- Piaget, J. The Moral Judgement of the Child — the paired-story method, still the clearest account of consequence-based reasoning.
- Vygotsky, L. "The role of play in development" — the dual-representation argument, in twenty pages.
- Wellman, H. The Child's Theory of Mind — the tasks, their order, and their limits.
- Winnicott, D. W. Playing and Reality — the clinical essay this week's prohibition is drawn from.
- Slade, A. "Representational play and the therapeutic process" — play as formulation rather than symptom.

Next: Week 6 — Middle childhood: competence, friendship and the school years.

Week 6 — Middle childhood: industry, peers, school

Developmental tasks. Industry versus inferiority (Erikson, roughly six to eleven); the consolidation of mastery through practice and product; the peer group as a structure with its own rules; a self-concept that becomes comparative ("I am good at maths, bad at football"); and moral reasoning that becomes reciprocal — fairness, turn-taking, and the beginning of genuine perspective exchange.

What changes in the family. The child's world widens and the parents' role changes from direct control to monitoring and scaffolding. School becomes a second system with its own rules, its own adults, and its own judgement of the child, and the family–school interface becomes a clinical site in its own right. A child now spends more waking hours with non-relatives than with relatives, which is new for the family and new for the child.

The developmental gift of this age. Middle childhood is the least dramatic period of development and the most consequential for identity, because it is when the child builds an answer to the question "what am I good at?" That answer will organise her

adolescence. The clinical stake is therefore not usually crisis; it is the slow accumulation of evidence — in a classroom, on a pitch, at a kitchen table — that the child is less than.

School refusal — a developmental and systemic differential. Ask: is this anxiety, separation difficulty, bullying, a learning difficulty, a family crisis, or a bid to protect a parent? The distinction matters because the same behaviour has opposite treatment implications.

> **CASE**
> A nine-year-old refuses school on Mondays only. Developmental enquiry: what happens on Sunday nights? The answer: the parents' weekly argument about money. The child's presence at home on Monday keeps the mother from leaving for work, and the argument stops. The symptom is a Monday-shaped solution to a marital problem — and it is also genuinely distressing to the child.

6.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The quiet age	Why middle childhood looks uneventful and is decisive	Recall what they were good at at nine, and who told them
8–22	Industry versus inferiority	The mechanics of mastery, and the four ways adults interrupt it	Identify the four from a case vignette
22–36	The comparative self	How "I am good at X" becomes "I am better/worse than Y"	Map a nine-year-old's self-description across five domains
36–48	Peers and friendship	The stages of friendship, and what a friend is for at seven, nine, eleven	Code two friendship descriptions against the stages
48–62	School refusal	The six-way differential, and the seven questions that decide it	Run the seven questions on the case and defend a formulation
62–74	Monitoring and scaffolding	The shift from control to oversight, and the crime of over-monitoring	Rewrite three controlling sentences as monitoring ones
74–84	The school as a system	How to work with a school without becoming its agent	Draft a letter to a head teacher
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students arrive believing that middle childhood is the easy placement, because the children talk and the problems are legible. The lecture has to revalue the age: the child is now being judged continuously by strangers, and the household's job is to interpret that judgement without either denying it or amplifying it. The single most useful teaching move is to ask the class what they were told they were bad at between the ages of seven and eleven, and to notice how many of them can still recite it in the original wording, thirty years later.

The sentence the lecture must land. "At this age a child is assembling an answer to the question 'what am I good at?', and she is using other people's faces as her evidence."

6.2 Five concepts that organise the week

1. Industry versus inferiority. The child now wants to make things and to be competent at them — a model, a goal, a sum, a drawing, a game of football, a recitation. The developmental gain is a self sustained by skill. The risk is inferiority: the settled conviction that one's efforts do not produce acceptable products. The mechanism is comparison, and the adults who shape it are teachers, coaches, siblings, and parents — in that order of hours, though not of weight.

The clinical translation is precise. When an eight-year-old presents as "lazy" or "not trying", the assessment question is not about motivation but about whether effort has ever produced a product that was received. A child who has never had the experience of making something and having it accepted will stop making things, and the behaviour that follows — avoidance, clowning, aggression, or quiet disappearance — is a rational response to a losing game.

2. The comparative self-concept. Before about seven, children describe themselves in absolute terms ("I can run fast"). After seven they describe themselves in comparative terms ("I am the third fastest in my class"), and they begin to describe themselves in the terms others use. This is the developmental moment when a family's labels become the child's self-description, and it is why the words used about a seven- to ten-year-old are worth more clinical attention than the words used about a toddler.

3. The peer group as a structure. The peer group is not a crowd of children; it is a system with rules, hierarchies, gatekeepers, and sanctions. Belonging is a developmental need, not a preference, and exclusion at this age produces measurable distress. Two clinical errors follow. The first is to dismiss peer difficulty as trivial ("children are cruel"). The second is to treat it as a problem to be solved by the therapist rather than by the adults who control the setting.

4. Moral reasoning becomes reciprocal. The child moves from "what will happen to me?" to "what is fair?", and fairness is understood as equal shares, turn-taking, and rules that apply to everyone including adults. This development is what makes this the age of the indignant question — "why does he not have to?" — and it is a resource for family work rather than an impertinence. A nine-year-old's sense of injustice is frequently accurate, and a family that can hear it gains a member who can name what is happening.

5. Monitoring and scaffolding replace control. Parents cannot see the child's day, so they must build a system of oversight: who is where, with whom, and how the child can reach an adult. Effective monitoring is warm, predictable, and interested. Ineffective monitoring takes two forms. Under-monitoring (no questions, no knowledge of the child's world) leaves the child unprotected. Over-monitoring (interrogation, phone tracking without conversation, no privacy) teaches the child to manage the surveillance rather than to use the adult, and it reliably produces the secrecy it is trying to prevent.

6.2a The school-refusal differential: seven questions

"School refusal" is a description, not a diagnosis, and it is the single most common developmental presentation of this age band. The differential has six members, and the questions below separate them in roughly fifteen minutes of interview if asked in this order.

Question 1 — When did it start, and what happened that week? An abrupt onset with a datable event points to a specific stressor: a fight, a humiliation, an exam, a bereavement, a change of teacher. A gradual onset over months points to an accumulating process: learning difficulty, bullying, or an anxiety that has been building.

Question 2 — Which days, and which lessons? A pattern is diagnostic information. Mondays only suggests a weekend–weekday transition and often a family process. One subject only suggests a specific failure or a specific adult. Mornings only, with recovery by break, suggests separation or anticipatory anxiety.

Question 3 — What happens in the body? Headache, stomach ache, vomiting, and sleeplessness before school, resolving once the child is allowed to stay home, is the pattern of anxiety. Symptoms that do not resolve on staying home point elsewhere — a medical problem, or a family in which being ill is the only available currency.

Question 4 — Is the child behind, and in what? Ask for the last report, not a description of it. A child who cannot read at nine will find a hundred ways not to be in a room where reading happens, and none of them will be called "reading".

Question 5 — Who is unkind, and where? Ask directly, and ask about the phone. Bullying at this age has moved partly into a space the school cannot see, and a child who is refusing school is often protecting the adults from information rather than protecting herself from school.

Question 6 — What is happening at home? Not "is there anything wrong at home?" — that question produces "no". Ask what the evenings look like, who is away, who is unwell, who is arguing, and what the child does when the arguing starts.

Question 7 — What does the child's absence make possible? This is the systemic question and the one students forget. Absence has consequences: a parent does not leave, a sibling is not alone, a grandmother is not left with nobody. Ask who benefits. The answer is rarely the child, and it is never nobody.

The formulation grid. Put the answers into five cells: anxiety, separation, school-based threat, learning, and family function. Most cases load on two. The treatment follows the loadings — an anxiety-driven case needs graded return plus parental alliance; a bullying case needs the school and protection before any exposure; a family-function case needs the family work first, because graded return will fail while the child's absence is doing a job at home.

6.2b What a friend is, at seven, nine and eleven

Friendship has stages, and each stage describes a different capacity the child brings to the clinic.

At six or seven a friend is someone you play with today. Friendship is concrete, provisional, and defined by shared activity. Betrayal is "he played with someone else". A child of this age who has no friends is not necessarily lonely in the way an adult means it.

At nine a friend is someone who is loyal, keeps promises, and shares. Friendship is now a relationship with obligations, and the characteristic injury is exclusion from a group rather than absence of a partner. This is the age at which "nobody sits with me" becomes a clinical statement.

At eleven a friend is someone who knows you, who you can tell things to, and who understands you without explanation. Intimacy and self-disclosure arrive. The characteristic injuries are betrayal of a confidence and the collapse of a friendship group, both of which present to clinics as sudden, dramatic, and — to parents — disproportionate.

The clinical use. Ask the child to describe a friend and listen for the stage. A nine-year-old who describes friendship entirely in terms of shared activity is not necessarily delayed, but a nine-year-old who describes no one at all is telling you about the setting. Then ask the parents the same question about their own friendships, because the household's model of friendship is usually the child's curriculum.

6.3 The method: assessing school refusal in one session

The method below is a sequence, and the order matters because each step changes the questions in the next.

Step 1 — Get the attendance data. Dates, not adjectives. "He refuses school" becomes usable when it is "absent eleven of the last twenty school days, all Mondays and two Fridays". Bring the register printout or the school's messages. Two clinicians looking at the same pattern will usually agree; two clinicians listening to two descriptions will usually not.

Step 2 — Interview the child alone, briefly, with the parents informed. Ten minutes, at the child's level, without the parents' faces. The two questions that produce most of the information are "what is the worst part of the morning?" and "what would happen if you went?" The second question is the one adults rarely ask, and it is the one that surfaces the feared consequence — a confrontation, a test, a parent alone in the house.

Step 3 — Separate the five cells. Write the five-cell grid in front of the parents and place the evidence in it. The purpose is not to be clever; it is to show the family that the behaviour has more than one possible meaning, which is itself an intervention in households that have settled on "he is being difficult".

Step 4 — Ask the systemic question aloud. "What happens at home when he stays?" Ask it plainly, and then be quiet. In the case below, the answer arrived in nine seconds.

Step 5 — Agree the return plan before the family leaves. Not "we will work on it". A graded plan with dates, a named adult at the school gate, a named person the child can go to at break, and an agreed consequence for the first morning. The plan is written down and copied to the school the same day.

Step 6 — Protect the plan from the family's other business. This is the step that fails. A return plan collapses when the underlying family process is unaddressed, and the therapist who has spent the session on the school will find the child back at home within a fortnight.

6.3a The family–school interface

The therapist is not the school's agent and not the family's advocate. The position is consultant to the system, and it is maintained by three practices.

Practice 1 — One channel. Agree with the family who speaks to the school, how often, and about what. Families with two parents, a grandmother, and a house help who all telephone the school produce a school that stops answering, and the child learns that the adults cannot coordinate.

Practice 2 — Ask for observations, not verdicts. A school says "he is disruptive". Ask what he did, in what lesson, after what, and what the teacher did next. Schools, like parents, describe children in adjectives; the therapist's job is to convert adjectives into sequences, in both directions.

Practice 3 — Give the school one thing to do. Not a programme. One change: a named adult at the gate, seating the child next to a specific person, a signal the child can use to ask for help without speaking in front of the class. A school that is given one workable action will do it; a school that is given a system will do nothing.

The letter. The letter below is the model taught in the laboratory, because a well-written letter is a clinical act and, in the experience of most trainees, the single most useful document they will write in their first year of practice. It is short, it names one request, it does not diagnose, and it does not blame.

> **PRACTICAL**
> Write the letter. Three paragraphs, 200 words, no jargon. Paragraph one: what the family has asked for help with, in one sentence. Paragraph two: what the clinic will do, and by when. Paragraph three: one specific request of the school, with a named person and a date. Bring it to the next session and read it aloud; the class will tell you the moment it becomes a complaint.

6.4 Three techniques for this age

Technique 1 — The week map. With the child, draw the seven days in a grid and put a mark on each day for how the morning felt, using a three-point scale the child invents. Then, in a different colour, mark the days something happened at home. The technique works because it is the child's own data, collected by the child, and it makes a pattern visible without anyone having to accuse anyone. It is also the beginning of the child's sense of agency: she is the one who noticed.

Technique 2 — The competence inventory. With the child, list five things she can do — not five things she is good at compared with others, five things she can do that she could not do a year ago. Then find one that an adult in the household does not know about, and

arrange for that adult to be told. The technique addresses the mechanism of inferiority directly: the child needs at least one domain in which effort produces a product that is received.

Technique 3 — The one adult. Ask the child to name one adult at school she would go to if something happened. If she cannot name one, that is the finding, and the school intervention is to create one. This single question predicts more about a child's resilience at school than any questionnaire, and it takes eleven seconds to ask.

6.4a The digital playground

By nine, a substantial minority of children in Nigerian cities are in a group chat with their classmates, and the therapy room has to take account of it because the peer group now has a second, invisible location. Three observations, and then what the therapist asks.

The group chat is a room with no adult in it. The developmental function is the same as the playground — belonging, status, exclusion — and the developmental difference is that it is continuous, archived, and unobserved. A child can be excluded at eleven at night, in her own bed, by forty classmates, and no adult will know unless she says so.

Exclusion at this age is often technical rather than social. A child is not told she is unwelcome; she is simply not added to the group. This is harder for parents to see and easier for a child to misinterpret, and it produces a particular kind of distress: the child knows something is happening and has no evidence.

What children do with a phone at nine is mostly not what parents fear. The realistic risks in this age band are not predation — which is real but rarer than the anxiety about it — but (a) the erosion of the boredom in which imagination happens, (b) the acceleration of comparison, because a nine-year-old now sees the edited highlights of forty lives, and (c) the collapse of the parental monitoring that this week's concept describes, because the child's social world has moved out of the parents' line of sight.

What the therapist asks, in this order. First: "does she have a device of her own, and where does it sleep?" The answer to the second half is more informative than the first. Second: "who is in her group, and does she want to be?" Third: "what happens in the house between seven and nine in the evening?" — because the family's evening is the variable that determines whether the device is a tool or a substitute.

What the therapist does not do. Do not run a session on screen-time limits. The evidence for specific hour-based rules is weak and the argument consumes the session. The two interventions with better support are structural and relational: no device in the bedroom overnight, for anyone in the household including the adults; and one daily period of twenty minutes in which the child has an adult's undivided attention, which is the same prescription as Week 4's ten minutes of child-led play, at a different age and with a different content.

The sentence that reframes the parental complaint. When a parent says "he is always on his phone", the developmental translation is usually "he has found a place where he belongs and it is not here". That translation is not always correct, but it is always worth testing, because it moves the intervention from confiscation to invitation.

6.5 The first session with a nine-year-old and two parents

Phase 1 — The child first (15 minutes), parents in the room. Nine-year-olds are usually relieved to be asked directly and embarrassed by being discussed. Ask the child: what is the worst part of the morning; what would happen if you went; who do you sit with; who would you tell. The parents hear the answers at the same time as the therapist, which prevents the later reconstruction in which the child's account is softened.

Phase 2 — The child out (5 minutes), with a clear explanation. "I am going to talk to your mum and dad about the grown-up parts. You are not in trouble and nothing you said was wrong."

Phase 3 — The parents (20 minutes). The seven questions, asked in order, with the attendance data on the table. The purpose is not to interrogate but to demonstrate the differential: by question five the parents usually begin to supply the formulation themselves.

Phase 4 — The systemic question (5 minutes). "What happens at home when he stays?" Ask it once, in plain language, and let the silence work. Then: "who is at home with him?" and "what would have to change for the mornings to be ordinary?"

Phase 5 — The plan (15 minutes). Written, dated, with names. One request to the school, one change at home, one thing the child will do. The parents leave with a page, not with an understanding.

Why the child goes first. Because a nine-year-old who has been discussed for twenty minutes before being seen will arrive defensive, and because the parents' account of a child's inner life at this age is frequently a description of the parents' own anxiety. The child's own words, heard by the parents in the room, do more to loosen a fixed formulation than an hour of parental interview.

6.5a When the child will not talk

A nine-year-old who has been brought to a clinic by two adults, about a problem the adults have defined, has three good reasons not to speak: she does not know what will happen to what she says, she has been told not to discuss family business, and she has learned that saying nothing is safer than being misquoted. The following five moves are taught because the standard one — "can you tell me how you feel?" — produces a shrug from a child of this age in almost every culture.

Move 1 — Give the child a job. "I need your help with something. I am trying to understand what a Monday morning is like in your house, and only you can tell me." A task recruits the child's competence, which is exactly what this stage is about, and it changes the child's position from subject to consultant.

Move 2 — Move the question to the third person. "Some children tell me the hardest part is the morning. Others say it is the night before. Which is it for the children you know?" The third person gives the child a legitimate way to describe her own experience without owning it, and it is not a trick: it genuinely produces different and often more accurate information.

Move 3 — Draw. A week map, a house with the rooms and who sleeps where, a line with marks for the good days. Drawing occupies the part of the child that would otherwise be managing the adult's face, and it produces a document the therapist and the child can both look at instead of each other.

Move 4 — Offer a scale with an escape. "On a scale of one to ten, how much does Monday morning worry you? One is nothing, ten is the worst." A number is easier than a sentence, and it can be revised. Children who will not answer a feeling question will answer a number, and the number is comparable across sessions.

Move 5 — Say what will happen to the information. Before the first question: "I will tell your mum and dad the things that are about the family, because they need to know them to help. If you tell me something that is not safe, I will have to tell them too. Everything else stays in this room." Nine-year-olds understand this perfectly, and the children who relax most visibly after it are the ones who had assumed everything was being reported.

What to do with silence. If the child says nothing for a full minute, wait. If the minute becomes four, stop asking and do the drawing yourself, narrating: "this is the Monday box, and I do not know what goes in it." Most children will correct the drawing, and a correction is the beginning of speech: it is the first time in the session that the child has decided the therapist's version is worth amending.

6.6 Case: the Okonkwos again — Mondays

The course's recurring family returns, six months after the sessions in Week 1, with a different presenting complaint about the same nine-year-old.

The referral. Ada telephones. "The defiance has stopped, but now he will not go to school. Only Mondays. Every Monday since the beginning of term."

The household. Ebuka, 9, in Primary 4. Ada, 34, a schoolteacher. Chuka, 39, a driver on night shifts, home from about five in the morning and asleep until two. The baby, now seventeen months. Chuka's mother died in 2023. Chuka's younger brother, 24, has been staying since December, sleeping in the sitting room, looking for work.

The attendance data. Eleven of the last twenty school days missed. All five Mondays; two Fridays in the same period, both following nights when Chuka was not on shift. Non-attendance on Mondays accounts for nine of the eleven.

The parent's account. "He says his stomach hurts. Sunday night he is fine, he eats, he watches football with his father. Monday morning he is crying. By ten o'clock he is playing. It is not real."

The child's account, ten minutes alone. The worst part of the morning is "when Daddy is coming in and Mummy is getting ready". What would happen if he went? A long pause. "They would be alone." Asked who "they" are: "Mummy and Daddy." Asked what happens when he stays: "Mummy doesn't go. She stays with me and we watch films. Daddy goes to sleep."

The seven questions, answered.

Question	Answer	Cell
Onset and date	Start of term, four weeks after the uncle moved in	Family function
Which days	Mondays, plus two Fridays when father was home	Family function / separation
Body symptoms	Stomach ache, resolves on staying home	Anxiety (secondary)
Is the child behind	Report: average in all subjects, excellent in mathematics	No learning problem
Who is unkind	Nobody named; one friend, Tobi, "he is my best friend"	No school-based threat
What is happening at home	Money; father's nights; uncle in the sitting room	Family function
What the absence makes possible	The mother stays; the parents do not argue; the uncle leaves the room	Family function

The Sunday-night mechanism. The interview with the parents, without the child, produces the mechanism in about four minutes. Sunday night is the only evening Chuka is reliably at home and awake. It is when the week's money is counted. For the last two months, the counting has not worked: the school fees for the term are short, the uncle has not found work, and Chuka has begun to talk about taking a second job on Sundays — which would mean he is never home in the evening at all. The conversation starts calmly and ends in an argument. Ada cries. Chuka leaves the room.

Ebuka hears all of it. He has done so since he was seven, because the flat has two rooms.

What Ebuka has constructed. A nine-year-old, at the stage where moral reasoning is reciprocal and causality is sought, has assembled a theory: the argument happens on Sunday night; when he is at home on Monday, his mother does not go to work, his father does not leave, and the house is quiet. He has therefore found a mechanism for preventing the argument, and he is operating it. He is not lying about the stomach ache; he is not fabricating it either. His body is doing what bodies do when a child has found that illness is the one currency that works.

The formulation, in one sentence. The symptom is a Monday-shaped solution to a marital and financial problem, and it is also genuinely distressing to the child — both things are true, and the therapist must hold both, because a formulation that makes the child merely strategic will produce an intervention that shames him.

Why this is not "manipulation". Manipulation requires representing another's belief and intending to change it for one's own advantage. Ebuka is not gaining anything a nine-year-old wants. He is paying: he misses his friend, he misses mathematics — the one thing he is excellent at — and he is alone in a quiet house with a sleeping father. The behaviour is a sacrifice, and the clinical error is to read a sacrifice as a con.

The intervention, in four parts.

Part 1 — Tell the child the truth, at his level. The therapist asks the parents to say three things to Ebuka, in the session, in their own words: that the money is short and it is the adults' problem; that they are not separating; and that he does not have to stay at

home to keep them together. Ada cries while saying it. Chuka says the third sentence twice. Ebuka says nothing and then asks whether he can go and play. That is the correct response.

Part 2 — Move the argument. Not "stop arguing" — an instruction no couple can follow. Instead: the money conversation happens on Saturday afternoon, out of the flat, in the car, and it does not happen on Sunday night. The content does not change; the time and the place do. This is a structural intervention, and it is the one that most reliably works, because it removes the child from the audience without asking the parents to change their marriage in a fortnight.

Part 3 — Give the Monday morning a shape. Ada leaves for work at the same time whether Ebuka goes to school or not. She does not stay. The handover to the uncle is explicit and cheerful: "Uncle will walk you at seven twenty." Chuka, coming in at five, sleeps in the inner room with the door closed, and the uncle is told, in front of Ebuka, that he is in charge of the school run. The Monday morning stops being a negotiation and becomes a timetable.

Part 4 — Give the father a Monday. Chuka takes Monday afternoon, from two until six, with Ebuka, once a week — football, or mathematics, or nothing. This is not a reward for attendance; it is the removal of the reason for the absence. The child was keeping the family together with his body. The father now does the keeping, on a timetable, in daylight.

6.6a Four weeks later

Attendance: nineteen of twenty days. One Monday missed, in the week Chuka's second job was confirmed.

Ada reports that Ebuka asked, on a Sunday evening, "is it Saturday tomorrow?" — meaning, is the money conversation today. She said no, it was yesterday, and he went to bed.

The stomach aches have not disappeared. They now occur on Sunday evenings rather than Monday mornings, which is a change the therapist regards as clinically meaningful: the symptom has moved from a device that altered the household's behaviour to an expression of ordinary anticipatory worry. The distinction between a symptom that does something and a symptom that says something is one of the most useful in this book, and it is the difference between the first and fourth sessions of this case.

Chuka's second job means he is away on Sunday afternoons. The Monday afternoon with Ebuka survives, on Tuesdays, at Ebuka's request.

What would have changed the plan. If the report had shown Ebuka two years behind in reading, the plan would have begun with the school and an assessment. If the child had named a child who was hurting him, the school would have been the first call and the family work second. If the attendance data had shown a scattered pattern with no day structure, the formulation would have moved toward anxiety and the intervention to a graded return with a named adult at the gate. And if the parents had refused to move the Sunday conversation — a real possibility, and a common one — the therapist would have

worked with the child alone on the sentence "the argument is not mine to stop", which is a smaller intervention with a smaller effect and is sometimes all that is available.

6.7 Transcript: telling a nine-year-old the truth

Twelve turns from the third session, with Ebuka present for the first half. The transcript is annotated for what the therapist is doing and, more importantly, for what she is not doing.

> **T1 Therapist:** Ebuka, your mum and dad told me something and I want them to tell you, because you have been working very hard on something and nobody told you what it was. — Framing. The child is credited before anything is disclosed, and the disclosure is handed to the parents, not performed by the therapist. **T2 Ada:** We have been arguing about money. On Sunday nights. You have heard us. — Plain, no euphemism. Note that she does not ask "have you heard us?" — the question would invite a denial and put the child in charge of the disclosure. **T3 Ebuka:** (nods, looks at the floor) **T4 Ada:** And I am sorry. It is not your fault and it is not your job. — Two sentences, both necessary. The apology is not for the argument; it is for the child having had to manage it. **T5 Therapist:** Ebuka, can I ask you something about Mondays? (he nods) What do you think would happen if you went to school? — Question 2, asked again in front of the parents so that they hear the answer, which is the whole point of the session. **T6 Ebuka:** (long pause) They would be alone. **T7 Chuka:** (quietly) We are not going anywhere. — The father's sentence. It is not a reassurance the therapist supplied; it is the one thing the child needed and could not ask for. **T8 Therapist:** Ebuka, say what you heard. — A checking move. Not "do you understand?", which produces a yes; "say what you heard", which produces the child's version and shows whether the message arrived. **T9 Ebuka:** That they are not going anywhere. And the money is not mine. — The child's formulation, in his own words, is the outcome measure of the session. **T10 Therapist:** The money is not yours. And here is the other thing. (to the parents) Tell him about Monday. — One more piece of information, again delivered by the parents. The therapist has said almost nothing of substance in this session, which is the technique. **T11 Chuka:** On Monday I sleep in the back room and Uncle takes you to school. And then in the afternoon it is you and me, every Monday, from two. — A timetable, not a promise. Concrete, dated, and owned by the father. **T12 Ebuka:** Can we do maths? — The child negotiates the content of the repair. This is the sign that he has accepted the new arrangement: he is now planning rather than preventing.

6.8 Laboratory: the school-refusal formulation

Aim. To produce a five-cell formulation and a written return plan from a single interview, within ninety minutes, and to defend the formulation against an alternative.

Procedure. The class divides into trios: therapist, parent, observer. The trainer supplies one of four vignettes, each of which loads on two cells. The therapist has thirty minutes for the seven questions, five minutes to draw the grid, and fifteen minutes to agree a plan. The observer scores against the rubric. Then the trio must argue the case for the alternative formulation, which is the part students find hardest and learn most from.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Data first	Accepted adjectives; no dates	Asked for the pattern but not the record	Obtained dates, days, lessons; used them to form hypotheses
The differential	Settled on one explanation in the first ten minutes	Considered two cells	Placed evidence in all five, named the loadings, and said what would change the formulation
The systemic question	Not asked	Asked, but answered by the therapist	Asked plainly, followed by silence, and the answer used
The plan	General advice	A plan with no dates or names	Dated, named, copied to the school, with a fallback
Position	Became the school's agent or the family's advocate	Held a position but said so	Held the consultant position and could describe how

The distinction that carries most of the marks. A formulation that cannot be falsified is not a formulation. The top band requires the student to state, in advance, what they would expect to see if they were wrong — "if this is family function, attendance should improve when the Sunday conversation moves, within three weeks; if it does not, I will re-open the anxiety cell."

6.9 Six errors

Error 1 — Believing the child. Not the child's honesty that is in question; the child's theory. A nine-year-old's account of why he cannot go to school is a genuine attempt at explanation by someone with limited information, and treating it as the formulation stops the enquiry at the first cell.

Error 2 — Disbelieving the child. The mirror error, and the commoner one in this age band. "He is manipulating me" closes the assessment. The child's stomach ache is real; the question is what it is for.

Error 3 — Making the school the client. Once the therapist's plan is "get him back to school", the family process becomes an obstacle and the therapist begins to coerce the family on the school's behalf. Attendance is an outcome measure, not the treatment.

Error 4 — Treating the symptom without moving it. A return plan imposed on an unaddressed family process produces a child who attends and deteriorates, or a child who attends for three weeks and then presents with something else. The mechanism has to be addressed, not just the behaviour.

Error 5 — Over-monitoring as a solution. Parents who respond to a child's withdrawal by interrogating, tracking the phone, and removing privacy will produce a child who is present and unreachable. The monitoring that works at this age is warm and interested, and it is built years before it is needed.

Error 6 — Ignoring the report card. A learning difficulty is the most under-detected member of the differential, because no child says "I cannot read" and no parent volunteers "he is behind". Ask for the document. Every time, and read it in the session rather than at home, because the parent's face while you read it is data you will not get twice.

6.10 Culture, faith and the child at school

Whose school is it? In many households the school is experienced as an extension of the state rather than of the family, and a child's difficulty at school is managed by the family without the school being told anything. A therapist who telephones the school without an explicit agreement will lose the family in one move. Ask first: "who in this family talks to the school, and what are they allowed to say?"

The child who is needed at home. Ebuka's case is not exotic; it is the ordinary structure of many households where a parent works nights, a relative is staying, and a nine-year-old is the most reliable person in the flat. The clinical question is never "is this child being parentified?" as an accusation, but "what is this child actually doing, and who else could do it?" The answer is frequently a specific task that can be reassigned to a named adult, and that reassignment is the intervention.

Faith and the meaning of difficulty. Households may interpret a child's trouble at school through prayer, deliverance, or a spiritual account of the family's season. A therapist who treats that account as a competitor to the assessment will be heard as an opponent. The workable position is to accept the household's framework as the language in which the difficulty is held, and to add a second language — what the child can do, what the school observes, what the report says — without asking anyone to give up the first.

The high expectations question. Many Nigerian and West African households hold a standard of academic achievement that is both a resource and a pressure. The resource is that the child's effort is noticed. The pressure is that a child who is average in a family of high achievers may conclude that she is inferior at eight. The clinical move is not to lower the standard but to widen the field: find the domain in which this child's effort produces a product, and make sure one adult receives it.

> **DEBATE**
> "A nine-year-old should be told the truth about the family's money, including the arguments." Argue for: the child is already hearing it, the silence is what produces the symptom, and the truth returns the burden to its owners. Argue against: children cannot hold adult information without cost, disclosure serves the parents' relief rather than the child's need, and "the truth" at nine is a version of the truth that someone has chosen. Then decide what you would actually say, word for word, to a nine-year-old whose parents cannot pay the fees.

6.11 Evidence note

What the research supports. The association between peer acceptance in middle childhood and later adjustment is one of the more robust findings in developmental psychopathology, with prospective studies showing that peer rejection at eight to ten predicts internalising and externalising difficulties into adolescence — though the effects are moderated strongly by the presence of at least one reciprocated friendship, which is the finding that matters clinically. A child with one friend is not a child without peers.

What is weaker. The evidence on school refusal is fragmented because the samples are heterogeneous: studies mix anxious children, oppositional children, and children absent for family reasons, and then report a single treatment effect. The best-supported component is not a therapy brand but a structural one — a rapid, coordinated return to school with parental involvement and a school-based adult — which appears in almost every effective protocol regardless of the theoretical label on it.

What is not supported. The claim that early intensive academic instruction in the preschool years produces durable advantage. The evidence points the other way: gains in directly instructed literacy and numeracy in the early years tend to fade, while the regulatory and social capacities that make use of instruction possible are the ones that persist. This is the empirical basis for the previous week's argument that school readiness is regulation.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same Monday refusal, in a household with a stable income and an available father, would have resolved without treatment in most cases. In this household, at this moment, it became the mechanism by which a family stayed organised.

6.12 Key terms

Industry versus inferiority — Erikson's fourth stage; the consolidation of competence, and the risk of a settled conviction of inadequacy.

Comparative self-concept — the shift, around seven, from absolute to relative self-description, and the beginning of self-description in other people's terms.

Monitoring — parental oversight of where, with whom, and how the child can reach an adult; effective when warm and predictable.

Scaffolding — support calibrated to current competence and withdrawn as competence grows.

School refusal — non-attendance driven by emotional distress or family process, distinguished from truancy (which is concealed) and from exclusion.

Graded return — a dated, stepped plan for re-entry, with a named adult and predictable supports.

Parentification — the assignment of adult responsibilities to a child; useful as a description and harmful as an accusation.

Reciprocated friendship — a mutual, named friendship, which moderates the effect of peer rejection more strongly than acceptance by the group.

Locus of control — the child's developing sense that outcomes follow from her actions; built at this age by tasks that are hard but achievable.

6.13 Self-test

1. A parent says "he is manipulating me". Give the definition of manipulation, and explain why it is usually the wrong word for a nine-year-old.
2. Name the five cells of the school-refusal formulation and one piece of evidence that would fill each.
3. Why is a symptom that does something different from a symptom that says something, and how does the distinction change treatment?
4. What is the difference between monitoring and over-monitoring, and what does each produce?
5. A child of nine describes friendship only as "we play football". Is this delayed? What else do you need to know?
6. Why does the laboratory rubric award marks for stating what would prove you wrong?
7. In the Okonkwo case, what would you have done if the parents had refused to move the Sunday conversation?
8. Name three things a school can do that cost nothing and change a child's week.

Answers. (1) Manipulation requires representing another person's belief and intending to change it for one's own advantage; at nine the capacity is emergent and the child in this pattern is usually sacrificing something rather than gaining, so the word misreads both the mechanism and the direction of the behaviour. (2) Anxiety, separation, school-based threat, learning, family function — evidence might be: body symptoms resolving on staying home; distress at the gate but not at the classroom; a named pupil or a phone message; a report showing a two-year gap; attendance data patterned by a day on which the child's presence changes the household. (3) A symptom that does something is functional within the system and will persist until the system's need is met elsewhere; a symptom that says something is an expression and responds to being heard and to the child's own coping. The first needs structural change, the second needs the child's distress addressed. (4) Monitoring is oversight with interest and warmth; over-monitoring is surveillance without relationship. The first produces a child who uses adults; the second produces a child who manages adults. (5) Not necessarily — at nine friendship is often described in terms of shared activity; you need to know whether there is a reciprocated friendship, whether the child is invited and refuses or is not invited, and what the parents' own friendships look like. (6) Because a formulation that cannot be falsified cannot guide treatment, and because stating the disconfirming evidence is the only protection against the therapist's preferred theory. (7) Work with the child alone on the sentence "the argument is not mine to stop", teach the parents one sentence to say to him before the argument begins, and re-open the case if attendance does not improve — a smaller intervention with a smaller effect, honestly labelled. (8) A named adult at the gate; a signal a child can use to ask for help without speaking in class; seating next to a specific child; a five-minute check-in at the end of the day.

6.14 Reflective task

Write down, in the original words if you can remember them, one thing an adult told you about what you were bad at between the ages of seven and eleven. Then write down the domain in which you were competent, and who noticed. Then answer: how many years did the first sentence take to stop being true? Most students find the second question harder than the first, which is the point.

6.15 Further reading

- Erikson, E. Childhood and Society — the industry stage, in the chapter on the school years.
- Harter, S. The Construction of the Self — the comparative self, and its measurement.
- Ladd, G. "Peer relationships and social competence" — rejection, friendship, and the moderating effect of one friend.
- Kearney, C. School Refusal Behavior in Youth — the functional model that underlies this week's grid.
- Selman, R. The Growth of Interpersonal Understanding — the friendship stages used above.

Next: Week 7 — Adolescence I: puberty, identity, autonomy.

Week 7 — Adolescence I: puberty, identity, autonomy

Developmental tasks. Identity versus role confusion (Erikson, from about twelve); pubertal change and its timing; separation-individuation without severance; peer belonging as a biological need rather than a preference; emerging sexuality; and the first genuinely ideological commitments — the adolescent's own answers to questions about religion, politics, loyalty and justice.

The neuroscience, briefly and correctly. The adolescent brain is not "broken": it is specialised. The reward system matures ahead of the regulatory system, which produces sensation-seeking and heightened sensitivity to peer approval alongside gradually improving impulse control that continues into the mid-twenties. Three things follow, and three things do not. What follows: adolescents are more responsive to peer presence than children or adults; they discount delayed outcomes more steeply; and they learn fast in conditions of high arousal and interest. What does not follow: that they cannot reason, that they are irrational, or that the findings license a lecture. On tasks with no peers present and no time pressure, adolescents reason as well as adults. The difference is in the conditions, and conditions can be designed.

The family task. To renegotiate the relationship: less control, more influence; more privacy, more accountability. Families that cannot renegotiate produce two common patterns: escalation, in which control increases as autonomy increases, and abdication, in which parents withdraw entirely and call it respect.

Why the two adolescence weeks are separate. This week is about the person — body, identity, autonomy, the developmental machinery. Next week is about risk — the specific

dangers that this machinery produces in this environment, and what a therapist does about them. The split is artificial and is made for teaching: in the clinic they arrive together, and the practitioner must hold both.

> **CASE**
> Amara, fifteen, in Senior Secondary 2. Her parents describe her as "secretive"; she describes them as "spies". The developmental reframe: privacy is a developmental need, and secrecy is what privacy becomes when there is no negotiated trust. The intervention is not more supervision and not more freedom; it is a negotiation — what will she share, what will they ask, and what happens when the agreement is broken?

7.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The problem with the question	Why "why is my teenager like this?" is the wrong question, and what replaces it	Write the question they actually want answered
8–22	Puberty	The sequence, the timing, and the two sexes' different relationships to it	Place four pubertal events on a timeline and mark the range
22–34	Timing effects	Early maturation in girls, late in boys, and the mechanisms	Predict the clinical presentation for each of four cases
34–48	The neuroscience	Four findings, three misuses, and the conditions under which adolescents reason well	Rewrite three "teenage brain" claims as testable statements
48–62	Identity	Marcia's four statuses, and how a therapist recognises each in a session	Classify four adolescent presentations
62–74	Autonomy without severance	The renegotiation, and the two failure patterns	Script a negotiation opening for a real family
74–84	Privacy and accountability	The three-column contract, worked on the whiteboard	Draft one contract for their own case
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students come to adolescence with either of two inherited attitudes: that teenagers are a problem to be managed, or that they are a mystery to be respected. Both are ways of not working with them. The lecture's job is to install a third attitude — that the adolescent is a colleague in a renegotiation, with legitimate interests and real obligations — and the way to install it is to have the class run the negotiation themselves, in role, against a parent played by the lecturer who does not concede easily.

The sentence the lecture must land. "The adolescent's job is not to leave you; it is to become someone you have not met yet, in front of you, while you are still paying for the food."

7.2 Five concepts that organise the week

1. Identity versus role confusion. Erikson's fifth stage, and the one that gives the age its name. Identity is not a feeling; it is a set of commitments — to a vocation, a set of beliefs, a kind of relationship, a place. James Marcia's operationalisation is the most useful tool in the week, because it turns identity into something observable in a session. He crossed two dimensions: exploration (has the young person actively tried options?) and commitment (has she settled on one?).

	Commitment made	No commitment
Exploration	**Achieved** — "I want to do physiotherapy because I worked in a clinic for a year and I liked the problem-solving"	**Moratorium** — "I am trying law, medicine and music; I will know by next year"
No exploration	**Foreclosed** — "I am going to be a doctor; my father is a doctor" (no examination of alternatives)	**Diffuse** — "I do not know, I have not thought about it"

The clinical value is that the four statuses have different prognoses and different treatments. Foreclosure is not a problem while it fits, and becomes one at twenty-six when it does not. Moratorium is healthy and anxiety-provoking. Diffusion in a fifteen-year-old is common, and diffusion at twenty-two with no exploration is a clinical concern. And the therapist's question is never "what do you want to be?" but "what have you tried, and what did you find out?"

2. Puberty as a social event as much as a biological one. The biological sequence is predictable: in girls, breast development, growth spurt, menarche; in boys, testicular growth, voice change, growth spurt. The social meaning is not predictable at all, and it does most of the clinical work. A girl who menstruates at nine in a school where the others are eleven will be treated as older than she is, by adults, for years. A boy who matures at sixteen in a culture that equates physical size with status will be treated as younger than he is, and may comply.

3. Separation-individuation without severance. The psychoanalytic language is clumsy and the phenomenon is real: the adolescent must become a separate person while remaining attached. The metaphor of leaving home is wrong for most of the world, where adolescents do not leave and are not expected to. What they must separate from is not the household but the household's definition of them — and they must do it in front of an audience that keeps feeding them.

4. Peer belonging as a need, not a preference. Peer presence changes adolescent risk-taking in experiments; being excluded activates the same systems as physical pain. This is not weakness; it is the developmental mechanism by which the young person moves their attachment from the family to the wider world. The clinical consequence: an intervention that requires the adolescent to be socially expendable will fail, and an intervention that uses the peer group will succeed more often than one that fights it.

5. Ideological commitment arrives. Adolescents begin to hold positions on justice, religion, politics and loyalty that are genuinely their own, and they test those positions against the adults in the house. The household experiences this as argumentativeness.

Developmentally it is the emergence of a moral agent, and it is the raw material of identity. A family that can survive being disagreed with produces an adolescent with convictions; a family that cannot produces either a secretive adolescent or a foreclosed one.

7.2a Pubertal timing: the effects that matter clinically

Timing, not just change, is a developmental variable with measurable consequences, and it is one of the few things in this book that a therapist can assess in a single question.

Early maturation in girls. Associated with higher rates of depressive symptoms, body dissatisfaction, and involvement with older peers — the last of which mediates much of the rest. The mechanism is not the hormones; it is the social response to a body that looks older than the person is. The clinical implication: a twelve-year-old who looks sixteen is being treated as sixteen by strangers, relatives, and sometimes by her own family, and she has the cognitive and emotional equipment of twelve.

Late maturation in boys. Associated with lower self-esteem and, in some cohorts, more risk behaviour in the late teens once maturation arrives — the "catch-up" pattern. The mechanism is again social: size is a currency in this age band, and a boy who does not have it is at the bottom of a visible hierarchy.

The reverse patterns are milder but real. Early-maturing boys gain social advantage; late-maturing girls generally do better on the outcomes that matter. Neither is a reason to be complacent: the advantage is contingent on the peer group's values, and it is the mismatch between the body's apparent age and the person's actual age that carries the risk.

What the therapist asks. One question, to the parent, in the child's presence or not: "when did you notice her body changing, compared with her friends?" And one to the adolescent, alone: "do people treat you as older or younger than you are?" The second answer is usually more informative than the first, and it frequently produces the material for the whole session.

The clinical presentations to expect. Early-maturing girls are over-represented in referrals for anxiety, mood, and "behaviour" that turns out to be an older peer group. Late-maturing boys are over-represented in referrals for withdrawal, "lack of confidence", and school refusal in the first years of secondary school. Both are frequently formulated as family problems before anyone asks about timing, and the reframe alone is often relieving for the adolescent, who has usually concluded that the problem is her character.

7.2b The teenage brain: four findings and three misuses

The neuroscience is useful to a family therapist only if it changes what she does in the room. It changes three things: how she explains risk to parents, how she designs tasks for adolescents, and how she thinks about the timing of consequences.

Finding 1 — The reward system is responsive, and more so in company. Mid-adolescents show heightened striatal response to reward, and this is amplified by the

presence of peers. Use: a plan that gives an adolescent something to gain, in front of people whose opinion matters, will outperform a plan built on avoiding loss.

Finding 2 — Regulatory control is still developing. Performance on inhibition and planning tasks improves steadily into the twenties. Use: do not build a plan that requires the adolescent to inhibit a strong impulse in a high-arousal moment; build one that removes the moment, or that puts an adult between the impulse and the act.

Finding 3 — Adolescents reason well under cool conditions. On tasks with no audience and no time pressure, adolescent reasoning is comparable to adult reasoning. Use: the therapy room is a cool condition, and an adolescent who is articulate and thoughtful in the session and reckless on Friday night is not being a hypocrite. She is reporting accurately from two different states, and the treatment target is the state, not the report.

Finding 4 — Sleep timing shifts. Melatonin release shifts later, so the adolescent is biologically not sleepy at ten and biologically not awake at seven. Use: this is the single most useful fact in the week for parents, because it converts "laziness" into a physiological fact — and because the practical recommendation (a consistent wake time, light in the morning, no bright screens in the last hour) is one that a family can actually follow.

Misuse 1 — "Their brains are not finished, so they cannot decide." The finding is about the rate of development of one system, not about competence. Adolescents can and must decide; the finding argues for scaffolding decisions, not for removing them.

Misuse 2 — "Peer pressure explains everything." Peers matter most for risk decisions in arousing conditions. They matter much less for the ordinary choices of a Tuesday afternoon, and an explanation that uses peer pressure for everything explains nothing.

Misuse 3 — "They will grow out of it." Some things resolve; some, like a depressive episode or a developing substance pattern, worsen. The neuroscience says nothing about which is which, and using it as reassurance in the wrong case is a serious error.

7.3 The method: renegotiating the frame

The adolescent session fails, when it fails, for one of two reasons: the therapist has been recruited as the parents' agent, or the therapist has tried to be the adolescent's friend. Both are avoided by negotiating the frame in front of both parties, in the first fifteen minutes, and writing it down.

Step 1 — Say what you have been told, and say it in the room. "Your parents have told me that you are secretive and that they are worried. They have told me some specific things, which I will not repeat in front of you without asking. I want to hear your version, and I will tell you afterwards what I think." The move is radical for families in which information flows upward only, and it buys the therapist the right to ask the next question.

Step 2 — Establish the confidentiality rule explicitly. The three-part rule: what the adolescent says stays in the room; exceptions are stated in advance (danger to self, danger to others, and — in most clinics — any disclosure of abuse); and anything about the household that the parents need in order to change is discussed with the adolescent

first, including how it will be said. The last part is the one that distinguishes a working adolescent practice from a performative one.

Step 3 — Ask what the adolescent wants out of this. The question is unfamiliar and usually productive. The answers cluster: "I want them to stop asking me the same question"; "I want to be allowed out"; "I want them to stop fighting"; "I want to be left alone". Each is a negotiable interest, and the therapist's next move is to convert it into a proposal.

Step 4 — Run the negotiation, not the therapy. With both parties present, three columns are filled: what she will share, what they will ask, and what happens if the agreement breaks. This is the substance of the session and it takes about twenty minutes. It works because it addresses the actual complaint — a broken information system — rather than the presenting emotion.

Step 5 — Forecast the test. The agreement will be tested within a fortnight. Say so: "someone will break this in the next two weeks. When it happens, we will look at it here rather than at home." This single sentence reduces the probability that the first breach becomes a family rupture, because it pre-frames the breach as data.

Step 6 — Give the parents a job they can do. Not "be less controlling" — unactionable. A specific behaviour: one question a day instead of six; a fixed time for the conversation; a weekly hour with no agenda. Parents who are given a specific behaviour change it; parents who are given an attitude do not.

7.3a The three-column contract, worked

The contract is a table on a sheet of paper with three columns, and it is the week's central practical.

Column 1 — What she will share, unasked. Typically: where she is, who with, when she will be back, and a message if she is late. These are safety data, not intimacy data, and separating them is the key move. An adolescent will usually agree to share safety data because it costs her nothing she values.

Column 2 — What they will ask, and how often. Typically: one question at a time, at a stated hour, and not at the moment she comes in the door. The specificity matters more than the content: "not in the first ten minutes after I get home" is a rule that can be kept, and it usually solves the complaint on both sides, because the parents' need is information and the adolescent's need is decompression.

Column 3 — What happens when it breaks. Written in advance, by both parties, and mild. A missed message means a phone call, not a grounding. A phone call means a conversation, not a punishment. The column exists because families without it escalate to the maximum sanction on the first breach, and the maximum sanction cannot be repeated, so it fails immediately.

The four things that are never in the contract. No blanket access to the phone's contents; no permission to read a diary; no requirement to report the content of friendships; and no agreement that the adolescent will "tell them everything". Each of

these is a demand for intimacy that the stage does not support, and each will be signed and then evaded, which is worse than not having an agreement at all.

What to do when the parents refuse the contract. This is common, and it is information rather than an obstacle. The refusal usually means the parents' own fear is unmanageable, and the therapist's next move is to take the fear seriously: "what are you most afraid will happen if she has a door you cannot open?" The answer is frequently specific and addressable — a pregnancy, a cult, a boy, a bus — and once it is named, a narrower agreement becomes possible.

7.4 Three techniques

Technique 1 — The five-domain identity interview. Ask, in order, about five domains: work (what are you good at, what do you want to be doing at twenty-five, what have you tried); beliefs (what do you believe that your parents do not); relationships (who matters to you and why); leisure (what do you do when nobody is watching); and body (how do you feel about how you look). Then classify against Marcia: exploration, commitment, both, neither. Twenty minutes, and it produces a formulation of the person rather than of the symptom.

Technique 2 — The question they cannot answer at home. "If your parents understood one thing about your life that they do not understand, what would it be?" Ask it in the joint session and then ask the adolescent to say it to them, not to the therapist. The technique fails about a third of the time and when it works it does more than four sessions of individual work, because the adolescent discovers that saying it is survivable.

Technique 3 — The future letter. Ask the adolescent to write, in ten minutes, a letter from herself at twenty-five to herself now. Read it in the session only if she offers it. The technique surfaces the content of identity — what she actually wants — which is otherwise unavailable to a fifteen-year-old who is asked directly, and it gives the therapist a vocabulary to use for the rest of the work.

7.5 The first session with a fifteen-year-old and two parents

Phase 1 — Both, briefly (10 minutes). Everyone in the room. The parents state the complaint in two minutes each; the adolescent is told she will get the same time and does. The therapist's only job here is to keep the time fair, visibly, because fairness is the currency of this age.

Phase 2 — The parents out (35 minutes). The adolescent alone. The confidentiality rule stated first, then the frame questions: what do you want out of this; what do they get wrong about you; who knows you best. Then the identity interview, or as much of it as she will give.

Phase 3 — The parents back (15 minutes). The therapist states, with the adolescent present, what she has permission to say. This is the hinge of the session: it demonstrates that the frame is real, and it usually produces a visible change in the adolescent's posture, because she has never seen an adult keep that promise.

Phase 4 — The contract (20 minutes). Three columns, written, signed by nobody (signing infantilises), photographed by everyone. The parents leave with a behaviour, the adolescent leaves with a rule, and the therapist leaves with a baseline.

Phase 5 — The follow-up question (5 minutes). "What would have to happen in the next two weeks for you to think this was worth coming to?" Ask both parties. Write the answers down. They are the outcome measures.

Why the adolescent goes second and gets more time. Because the parents have had fifteen years of being listened to and the adolescent has had none, and because the therapeutic alliance in this age band is the entire treatment. A therapist who spends the session with the parents and fifteen minutes with the adolescent has built a relationship with the wrong client.

7.5a When the adolescent will not speak

The silent adolescent is the commonest difficulty in this age band, and the standard response — waiting patiently, then asking about school — produces forty-five minutes of nothing. Five moves, in order of escalation.

Move 1 — Name the situation honestly. "You have been brought here by two people who are worried about you, and you did not choose to come. I would not want to talk either." This is not a technique for building rapport; it is an accurate description, and adolescents are unusually responsive to accuracy.

Move 2 — Ask about the thing she is allowed to have opinions about. Not her feelings. Her views: what is wrong with the school, which teacher is useless, what her parents do not understand about her generation. The identity stage runs on judgement, and an adolescent who will not describe herself will happily describe the world.

Move 3 — Use a third object. A phone (theirs, not yours), a diagram, a list of statements to sort. "The things people say about teenagers are on these cards. Sort them into true and rubbish." Every card sorted is a self-description the adolescent has given without being asked for one.

Move 4 — Offer a deal with a real cost to you. "Give me ten minutes of real answers and I will spend the last twenty minutes of this session arguing your case to your parents, in front of you." This works because it is a trade between parties with interests, which is the register of the age, and because it is kept.

Move 5 — Accept the silence and change the client. If none of that works, end the individual session early and say why: "I am not going to sit here and pretend. I will work with your parents for now, and the door stays open." This protects the alliance for the session in which she does talk, which is often the fourth.

What never works. Asking "how are you feeling?" three times; interpreting the silence ("you seem angry"); producing a worksheet; or telling the adolescent that she is "resistant". The last is the most common and the most damaging, because it converts the therapist's failure into the client's defect — and the adolescent, who has been described in deficit terms by adults her whole life, will accept the description and stop coming.

7.6 Case: Amara, secretive and spied upon

Referral. The Adeyemis, whom the course first met in Week 3, return. Amara is fifteen and in Senior Secondary 2. Her mother, Funke, 42, telephones: "She has a boyfriend and she is lying to us. We found the phone. Her father is coming home on Friday and he wants this finished."

The household. Tunde, 47, an engineer on a contract in Lagos, home two weekends in four. Funke, a school administrator. Segun, twelve. A paternal aunt, 55, who lives with them and who, Funke says, "knows everything before I do".

The parents' account. Six weeks ago Funke looked through Amara's phone while she was in the bathroom. She found a message thread with a boy in SS3, and a second thread in which Amara described her mother to a friend as "the police". Since then: the phone is confiscated at nine; Amara's movements are checked by the aunt; the gate is locked at seven; and Amara has stopped speaking at dinner. Funke's summary: "she is secretive, and she is becoming somebody I do not know."

Amara's account, alone, in the second session. "They are spies. She reads my phone. Auntie reports where I go. My father comes home and shouts for two days and then leaves and it is worse." Asked what the messages were about: "He is in SS3. We talk. That is all." Asked what she wants out of this: "I want a door."

What the therapist establishes, in three questions. First: what have you tried that they did not notice? Answer: she has started walking home the long way, and she has begun attending a church youth group on Wednesdays that she did not tell them about, "because if I tell them they will come". Second: what do you want to do after school? A long pause, then: "Physiotherapy." Who decided that? "Me. I read about it." Does your father know? "He says I am doing medicine." Third: when did you last tell your mother something true? "When I was thirteen."

The three findings, in order of clinical weight.

1. The parents' surveillance is functioning as designed and producing the opposite of its aim. Phone search, aunt as informant, locked gate, no negotiated agreement. Each measure was a response to a real fear, and together they have produced an adolescent with a private life she now has a reason to hide. This is the mechanism of the week's reframe: secrecy is what privacy becomes when privacy is not available.

2. The identity work is real and is being read as defiance. She has made an occupational commitment (physiotherapy, based on exploration — she read about it, she talked to a physiotherapist at the church clinic) and a belief commitment (the youth group). Her father has foreclosed the first on her behalf: medicine, because he is a man who did not have the choice himself. The conflict at the dinner table is not about a phone. It is about whether she is allowed to be the author of her own life, and the father experiences the question as ingratitude.

3. There is nothing in the referral that constitutes a safeguarding concern. The boy is seventeen, the relationship is described as talking, there is no coercion, no age gap that triggers the clinic's protocol, and no indication of harm. The therapist records this

explicitly, because the family's escalation invites a professional to escalate with it, and a clinician who over-reacts here will be used as an instrument of the surveillance.

The formulation. A fifteen-year-old at the moratorium stage, with one commitment made and one foreclosed by her father, in a household where the information system has broken and been replaced by surveillance. The presenting problem is the broken system. The underlying problem is that the father's absences have left the mother managing alone, with an aunt as a second authority and no agreed hierarchy — which is the Week 3 problem in this family, returned at a different age.

The intervention, in four parts.

Part 1 — Restore an information system, on the three-column contract. Safety data unasked (where, who, when, a message if late). Questions at one stated hour, not at the door. A breach met with a phone call, not a lockdown. The phone returned, with the agreement that the parents do not read it — and with the therapist saying plainly, in front of the adolescent, that she has advised this and why: because a fifteen-year-old with no private space will create one, and the created one is the one you cannot see.

Part 2 — Give the aunt a defined role, aloud. The aunt is not the problem; the undefined authority is. Funke and the aunt agree, in the session, on what the aunt decides (whether Amara goes out at all while Funke is at work, and safety) and what she reports (arrival times only, and anything unsafe — not friendships, not clothes, not moods). The sentence is said to Amara: "Auntie is in charge of whether you go and when you come back. She is not in charge of who your friends are."

Part 3 — The Wednesday. Amara tells her parents about the youth group, in the session, and the parents agree that she attends without being accompanied. This is the cheapest and most powerful intervention in the case: a piece of her life is voluntarily disclosed, in exchange for nothing, and its effect on the parents' fear is larger than any explanation the therapist could give.

Part 4 — The father's two days. Tunde's fortnightly returns are the family's structural problem. In the session with him present, the therapist asks one question: "what do you want her to remember about these two days, in ten years?" He is silent for a while and then says: "that I asked her things." The agreement that follows is specific and small: on the Saturday of each return, one hour, no agenda, no school results, no phone — and the topic is hers. The therapist does not ask him to accept physiotherapy. She asks him to ask her about it, in front of her, once, and then to say what he thinks without deciding.

7.6a Six weeks later

The contract has been broken twice. The first time, Amara came home at nine-forty without a message; the phone call was made; the conversation lasted four minutes; nothing was confiscated. The second time, Funke asked a question at the door; Amara said "not now, at eight", and the question was asked at eight.

Amara has told her father about physiotherapy. He has not agreed. He has stopped saying "you are doing medicine", which the therapist logs as the realistic outcome of a single hour, and which is more than the referral predicted.

The aunt has been given her defined role and has, according to Funke, "started telling me less", which Funke reports with ambivalence and which is the correct direction.

The most informative change is one nobody planned. Amara has started asking her mother about her own school days. Funke reports this as a curiosity; the therapist hears it as the beginning of the adolescent's interest in the parent as a person, which is the developmental signature of the move from identity to intimacy that occupies the late teens.

What would have changed the plan. If the messages had contained sexual content with an adult, the safeguarding protocol would have taken precedence over everything in this section. If Amara had described coercion, threats, or a pregnancy, the plan would have been safety first and negotiation second. If the father had refused to attend, the work would have proceeded with mother and daughter, with a specific question kept open for him. And if Amara had refused the contract entirely — a real possibility — the therapist would have worked with the parents alone on the surveillance, on the grounds that the system that is producing the secrecy does not require the adolescent's consent to be changed.

7.7 Transcript: the negotiation, with the contract on the table

Twelve turns from the third session. Tunde is present for this session, having flown in the night before. The adolescent is fifteen. The therapist has a blank three-column sheet in front of her and a pen she is not holding.

> **T1 Therapist:** I want to do something different today. I want us to write three lists, together, and I want the lists to be the only thing we argue about. The first list is what Amara tells you without being asked. (to Amara) You start. What would you be willing to tell them without being asked? — The therapist hands the first move to the adolescent, which is the whole architecture of the intervention. Note what she does not do: she does not begin with the parents' complaint. **T2 Amara:** Where I am. When I am coming back. If I am late. **T3 Tunde:** That is not the problem. The problem is — **T4 Therapist:** (gently) Hold it for four minutes. I promise it gets a column. — The single most useful sentence in adolescent family work: a promise of a specific place for the objection, with a time limit. It works because it is true. **T5 Tunde:** (nods, sits back) **T6 Therapist:** (writing) Where you are, when you are coming back, a message if you are late. That is column one. Second list: what they get to ask, and when. (to the parents) What do you need to know? **T7 Funke:** Who she is with. And where. **T8 Therapist:** "Who she is with" is in column one, if she will put it there. (to Amara) Will you? — Note the move: the therapist does not grant or refuse the request; she routes it to the person who owns it. This is what "negotiation" means operationally, and it takes about eight seconds. **T9 Amara:** (pause) Yes. But not the names. Just "a friend from school". **T10 Funke:** How is that supposed to — **T11 Therapist:** (to Funke) That is a counter-offer, and it is the first one she has made. Take it or make one, but do not lose it. — The therapist names the offer as an offer, which changes its status from a half-victory to a move in a negotiation. Parents in these sessions consistently miss the conciliatory move because it arrives in the same tone as the defiant one. **T12 Tunde:** (to Amara, not to the therapist) If you tell us "a friend from school" and you are late, what happens? — The father has moved from complaint to terms. This is the turn the session was for, and the correct response is to let him run it.

7.8 Laboratory: the negotiation, marked

Aim. To conduct a three-column negotiation with an adolescent and two parents in twenty-five minutes, and to keep the frame intact when it is tested.

Procedure. Quadruples: therapist, adolescent, two parents, one observer. The trainer supplies a one-page brief to each role privately, including one instruction designed to break the frame — the adolescent is told to refuse column one entirely; one parent is told to demand the phone's contents; the other is told to leave the room if the therapist takes the adolescent's side. The therapist must run the negotiation without knowing which brief is which. Twenty-five minutes, then the observer reports which pressure arrived and how it was handled.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The frame	Confidentiality not stated, or stated and then broken	Stated clearly, with exceptions, but not held under pressure	Stated, held, and used — the therapist refuses to report content and says why
The three columns	Two or fewer filled, or filled by the therapist	Three filled, but with unactionable items	Three filled, specific, with breach consequences written by the parties

Criterion	0–1	2–3	4–5
Voice	Therapist speaks more than the family	Therapist speaks about as much	Therapist speaks least; both parties address each other
Handling refusal	Concedes the frame or abandons the session	Holds the frame, but argues	Holds the frame, names the refusal as information, and offers a narrower agreement
The close	Ends with understanding	Ends with a plan	Ends with a plan, a forecast of the breach, and each party's own outcome question

The distinction that carries most of the marks. The top band requires the student to not fill in a column themselves. A therapist who says "so you will tell them where you are" has written the agreement on the family's behalf, and the family will not keep a therapist's agreement.

7.9 Six errors

Error 1 — Becoming the parents' agent. The referral was written by the parents; the therapist reads it; the first session is with the parents. By the time the adolescent arrives she is meeting an employee. The correction is procedural: the adolescent is seen first, or at least given equal time and equal billing, in the first session.

Error 2 — Trying to be the adolescent's friend. The opposite error, and it produces a therapist the adolescent does not respect and the parents do not trust. The position is not friendship; it is advocacy with limits — the therapist is on the adolescent's side in the sense that she will not lie to her, which is a different and more durable thing.

Error 3 — Treating the phone as the problem. The phone is the site. Remove it and the site moves. The formulation is about an information system and a developmental need, and a therapist who negotiates about a device has accepted the family's definition of the problem.

Error 4 — Calling normal development a disorder. Moratorium looks like confusion; experimentation looks like risk; argument looks like opposition; a late bedtime looks like defiance. Each has a developmental reading, and the diagnostic threshold in this age band should be high — which is not the same as absent.

Error 5 — Colluding with cultural foreclosure. "In our culture children do what their parents decide" may be true as a description of a household and false as a description of a fifteen-year-old's development. The therapist's job is not to overturn the household's values but to make room for the question of what the young person actually wants, because a foreclosed identity chosen for someone else is a stable arrangement that fails at twenty-six.

Error 6 — Ignoring the parent who is absent. Tunde's two days in four are the structure of this family and no one in the referral mentioned them. The absent parent's absence is a clinical fact even when — and especially when — it is presented as a virtue.

7.9a A seventh error: the one that ends treatment

There is a seventh error in this age band, and it is not a technique error; it is a relationship error with a predictable end.

Error 7 — Becoming the only adult who understands her. The therapist who says, or implies, "I am the one person who gets you" has made a promise that requires the family to remain incomprehensible. It is flattering to the therapist, it is common in trainees working with adolescents, and it converts the work into a competition in which the adolescent must choose between the therapist and her parents — a choice that will be made against the therapist the moment the therapy ends.

The correction is a discipline, and it has three parts. Never criticise the parents in the adolescent's presence, even when she invites it and even when the criticism is accurate. Always attribute the adolescent's insights to her rather than to your own cleverness ("you worked that out", not "as I said last week"). And always be explicit that the aim is a better relationship at home, not a better relationship with you: "if in six weeks you and your mother can have this conversation without me, we have done the job."

The marker of the error in supervision is a therapist who reports that the parents are "the problem" and that the adolescent is "the only sensible one in the family". The marker of the correction is a therapist who can say what she likes about each member and can name, without contempt and without exception, the interest that each of them is pursuing in the room.

7.10 Culture, faith and the adolescent

Dating, courtship and the family's stake. In many households the adolescent's relationships are not private matters but family matters, because marriage is a family project and a daughter's reputation is a family asset. A therapist who frames the issue as "her right to privacy" will lose the parents in one sentence, and will have done nothing for the adolescent. The workable frame is the one this week uses: safety data belongs to the household, intimacy data belongs to the young person, and the boundary between them is written down. That frame is defensible inside the household's own values, which is why it is usable.

Faith as an identity resource. For many adolescents the first genuinely owned commitment is religious, and the youth group, the choir, or the mosque's youth programme is the setting in which identity work happens. Amara's Wednesday is the ordinary case. A therapist who hears "church group" as a symptom of parental control will misread the most developmentally healthy thing in the referral.

Gender and the double standard. Households frequently hold different rules for sons and daughters, and the adolescent will name this before the parents do. The therapist should neither endorse nor dismiss the double standard; she should make it speakable, because a rule that cannot be discussed becomes a rule that is broken. The question that tends to work: "what is this rule protecting, and is there a version of it that everyone can live with?"

The aunt, the uncle and the second authority. Extended-household caregiving is a resource and a coordination problem. The clinical rule from Week 3 applies unchanged: authority must be defined aloud, in domains, in front of the child. A household with three adults and no defined hierarchy produces an adolescent who plays the adults off against each other, and the adults will describe that as manipulation.

> **DEBATE**
> "A parent has the right to read a fifteen-year-old's messages." Argue for: the parent is legally and morally responsible for a minor's safety, the child has no expectation of privacy in a phone the parent paid for, and the risk of not knowing is severe and irreversible. Argue against: reading messages purchases information at the cost of the relationship that would have produced it, the adolescent will simply move to a second account, and the intervention makes the parent the adversary at exactly the age when the adolescent most needs an adult ally. Then state your actual rule, and say what you would do if a parent told you they had already read the messages and found something alarming.

7.11 Evidence note

What the research supports. The dual-systems account of adolescent risk — a relatively mature reward system and a still-developing control system — is well supported, and the peer-amplification finding has been replicated in both laboratory and naturalistic designs. The identity-status literature is observational but consistent: exploration with subsequent commitment (achieved) is associated with better adjustment than foreclosure or diffusion, across a wide range of cultures, though the content of identity differs by setting.

What is weaker. The claim that parenting style causes adolescent outcomes is confounded by the obvious possibility that adolescents influence parenting. The longitudinal work that separates the directions finds bidirectional effects, with parental warmth and monitoring predicting somewhat better outcomes and adolescent behaviour predicting parental monitoring strongly. The practical implication: a parent who monitors more because the adolescent is in difficulty is doing something reasonable, and the question is whether the monitoring is warm and negotiated or surveillance.

What is not supported. That adolescent-parent conflict is inherently pathological, or that its absence is health. Cross-cultural work finds that moderate conflict over autonomy is normative in settings where adolescents have individualised futures, and that the families with no conflict at all are as likely to be foreclosed as harmonious.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same parental monitoring that protects a thirteen-year-old can damage a seventeen-year-old, and the therapist's judgement is about calibration rather than quantity.

7.12 Key terms

Identity versus role confusion — Erikson's fifth stage; the construction of commitments to work, belief, relationship and place.

Moratorium — active exploration without settled commitment; developmentally healthy and anxiety-provoking.

Foreclosure — commitment without exploration, usually inherited from the family.

Diffusion — neither exploration nor commitment.

Separation-individuation — becoming a separate person while remaining attached; in most of the world this happens inside the household rather than by leaving it.

Pubertal timing — the developmental effect of when maturation occurs relative to the peer group, independent of maturation itself.

Dual-systems model — the account of adolescent risk as the product of an early-maturing reward system and a later-maturing control system.

Monitoring — parental knowledge of the adolescent's world; effective when warm and negotiated.

Surveillance — information obtained without the adolescent's participation; reliably produces concealment.

Negotiated trust — an explicit, written agreement about what is shared, what is asked, and what happens when it breaks.

7.13 Self-test

1. Give Marcia's four identity statuses and one presentation of each that a therapist would meet in a first session.
2. A twelve-year-old girl who looks sixteen is referred for "defiance". What is your first hypothesis, and what one question tests it?
3. State the dual-systems model in two sentences, and name one thing it does not license.
4. Why does the therapist in the transcript promise the father four minutes, and what would have happened without the promise?
5. Distinguish monitoring from surveillance, and give one behavioural example of each.
6. An adolescent says nothing for the first twenty minutes. Name three moves, in order.
7. What are the three columns of the contract, and what are the four things that must never appear in it?
8. In the Adeyemi case, what was the single cheapest intervention, and why did it work?

Answers. (1) Achieved — "I want to do physiotherapy because I worked in a clinic and liked it"; moratorium — "I am trying law, medicine and music"; foreclosed — "I am doing medicine, my father is a doctor"; diffuse — "I do not know, I have not thought about it". (2) That the social response to an early-maturing body is doing the clinical work — she is being treated as older than she is; the question is "do people treat you as older or younger than you are?" (3) The reward system matures ahead of the control system, so sensation-seeking and peer sensitivity are high while inhibition is still developing; it does not license the claim that adolescents cannot reason or decide. (4) To give the objection a specific place, so that the father could tolerate being interrupted without concluding he

had been silenced; without it he would have escalated, and the negotiation would have become a contest the therapist had to win. (5) Monitoring is knowledge acquired with the adolescent's participation and is warm; surveillance is knowledge acquired without it. Example of monitoring: a stated evening hour in which the day is discussed; of surveillance: reading the messages. (6) Name the situation honestly; ask for views rather than feelings; use a third object or a trade with a real cost to the therapist. (7) What she will share unasked; what they will ask and how often; what happens when it breaks. Never: blanket access to the phone, reading a diary, reporting the content of friendships, or a promise to tell them everything. (8) The Wednesday youth group disclosed in the session — it cost the adolescent little, gave the parents a piece of her life voluntarily offered, and reduced their fear more than any explanation could.

7.14 Reflective task

Think of one thing you decided about yourself between thirteen and sixteen that your family did not know at the time. Write it down, without naming anyone. Then answer three questions: what would have happened if they had found out? What did keeping it cost you? And what would an adult have had to do to be told? Most students find that the third answer is very specific and very small — a particular aunt, a particular drive, a particular hour of the evening — which is the point: adolescents do not withhold from adults in general, they withhold from adults who have not earned a particular door.

7.15 Further reading

- Erikson, E. Identity: Youth and Crisis — the source text, still the clearest.
- Marcia, J. "Identity in adolescence" — the four statuses and the interview that produces them.
- Steinberg, L. Adolescence — the dual-systems account, with the evidence.
- Smetana, J. "Adolescents and parents: negotiating autonomy" — the conflict literature, and why moderate conflict is normative.
- Keddie, A. and Dinham, J. on extended-family authority — useful on the second-authority problem in households with resident relatives.

Next: Week 8 — Adolescence II: risk, peers, digital life and mental health.

Week 8 — Adolescence II: risk, peers, digital life and mental health

Risk behaviour is normative in moderation and dangerous in combination: substance use, risky driving, unsafe sex, self-harm, disordered eating. Risk is not a moral category; it is a developmental phenomenon shaped by peers, family, school and access. The purpose of this week is to replace the question "why is he doing this?" with the four questions that actually predict harm: what, how often, with whom, and what does it do for him?

Digital life. Assess function, not just quantity: what is the phone doing in this adolescent's life — connection, escape, status, self-soothing, harm? Ask about sleep, cyberbullying, sexual content, and the family's rules about devices. Banning without

substituting an alternative rarely works, and the substitute is always a relationship rather than an activity.

Mental health. Half of all lifetime mental disorders begin by mid-adolescence. Screen, do not guess. Distinguish transient distress from disorder by **duration, impairment and severity** — and use the same developmental question as every other week: is this a normal task being struggled with, or a disorder?

What makes this week different from the last. Last week was the person. This week is the environment the person is in — the peer group, the phone, the substances that are available at the junction, and the adults who are or are not watching. The clinical skill is to hold both: an adolescent who is self-harming is a person in developmental transition and a person in an environment, and a formulation that has only one of those halves will produce an intervention that fails.

> **PRACTICAL**
> Conduct a twelve-minute adolescent interview with a peer who has been briefed to be reticent. You may not ask more than two questions in a row without reflecting something back. The observer records the ratio of questions to reflections, and whether the adolescent elaborated. The target ratio is one reflection to every two questions.

> **CAUTION**
> This week contains material on self-harm and suicide. The clinical content is included because a family therapist who cannot ask the questions directly will miss the presentations that matter most. The teaching position is that asking does not plant the idea; the evidence on that point is consistent and should be stated explicitly to every class and to every parent who raises it.

8.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	Three questions that replace one	Why "why is he doing this?" produces nothing usable	Rewrite the question for two cases
8–22	Risk as development	The four risk domains, their functions, and the combinations that matter	Plot four vignettes on a function-versus-harm grid
22–34	Peers and access	Peer amplification, and the local availability map	Draw an availability map for their own catchment
34–48	Digital life	Assessing function rather than hours; sleep, bullying, sexual content, and the substitute	Take a digital history from a volunteer
48–62	Mental health	Onset, the duration-impairment-severity rule, and the seven presentations not to miss	Screen two vignettes and justify the referral
62–74	Self-harm and suicide	What to ask, in what words, and what to do with the answer	Rehearse the four questions aloud, in pairs

Minutes	Segment	What the lecturer does	What students do
74–84	The family's response	Escalation, abdication, and the third option	Script a family safety plan
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. The lecture must rehearse the suicide questions aloud, in the room, by every student, because the barrier to asking is entirely in the throat. A therapist who has never said "have you thought about killing yourself?" out loud will substitute a euphemism at the moment it matters, and the euphemism does not get answered. The rehearsal is uncomfortable and it is the most important ten minutes in the module.

The sentence the lecture must land. "Risk behaviour is a solution somebody is using. Your first job is to find out what problem it solves, and your second is to find out whether it is working."

8.2 Five concepts that organise the week

1. Risk is a developmental phenomenon, not a moral one. Sensation-seeking, peer sensitivity, and a steep discounting of delayed outcomes are features of the period, not defects of the person. The clinical consequence is that prevention works better than prohibition: an adolescent who is given a domain in which risk is supervised — sport, performance, competition, a demanding apprenticeship — takes less unsupervised risk, because the developmental appetite has somewhere to go.

2. Function beats frequency. The question is not how much but what for. Cannabis every weekend with a group of friends who play football is a different clinical object from cannabis alone at eleven at night. The five functions that account for most adolescent risk behaviour are: belonging (the group does it); escape (it makes the evening bearable); status (it changes how others see me); self-soothing (it quiets something); and self-punishment (it expresses what I cannot say). Each function points to a different intervention, and the therapist who has not identified the function is guessing.

3. Combination is the danger, not any single behaviour. One risk behaviour in an adolescent with a stable household and an available adult is usually a phase. The same behaviour combined with school withdrawal, sleep loss, a bereavement, and an absent parent is a different prognosis. The assessment should therefore always ask: what else is happening at the same time? The answer is the risk multiplier.

4. Digital life is a setting, not an activity. The phone is where the peer group lives, where status is measured, where sleep is lost, and where a substantial proportion of bullying and sexual content arrives. Assessing "screen time" tells you almost nothing. Assessing what the phone is doing — connection, escape, status, self-soothing, harm — tells you what to do. The single most useful structural question is where the phone sleeps.

5. Most adult mental disorders begin here. The epidemiological finding is robust: half of lifetime disorders have onset by the mid-twenties, with anxiety, mood, substance and impulse-control disorders beginning earliest. This makes adolescence the period of

maximum preventive leverage and the period in which the family therapist is most likely to be the first clinician in the room. The obligation that follows is a screening obligation: a therapist who is working with an adolescent for anything must be able to ask about mood, anxiety, sleep, eating, self-harm and substances, and must know where to refer.

8.2a Seven presentations that must not be missed

The differential in this age band is wide and the presentations are quiet. The seven below are the ones that a family therapist must be able to recognise, name, and refer — not diagnose.

1. Depression. Not "sadness". The adolescent pattern is more often irritability than sadness, combined with withdrawal from things that used to matter, sleep change, appetite change, a fall in school performance, and a flatness that the family describes as "laziness". Duration of two weeks or more, with impairment, is the threshold that warrants a referral. One question screens surprisingly well: "when did you last enjoy something?"

2. Anxiety, including panic and social anxiety. Physical symptoms — racing heart, breathlessness, nausea — presenting as school refusal or as repeated visits to a clinic for a physical complaint. The distinguishing feature is anticipatory fear that leads to avoidance, and the avoidance is what maintains it.

3. Psychosis prodrome. Social withdrawal, a drop in functioning, odd ideas that are treated as eccentricity, suspiciousness, a decline in self-care, and sometimes a preoccupation with a new interest that has become the whole of the person's life. This is rare and it is the presentation where delay costs most. Any adolescent with a marked, unexplained decline in functioning plus unusual beliefs should be referred for assessment, promptly, and the referral should not be softened into "stress".

4. Substance use. Ask about what, how often, with whom, and what happens when it stops. The developmental danger signs are: use alone, use before midday, use to manage sleep or mood, and withdrawal from other activities. Do not accept "only at parties" without asking what happens at parties.

5. Disordered eating. Weight change is the least reliable sign. More useful: eating in secret, food rituals, a sudden interest in "healthy" eating that narrows to a list, exercising alone and compulsively, and a change in the family's mealtimes. The family's report of "she has become fussy" is worth three questions.

6. Self-harm. Cutting, burning, hitting, and — increasingly — the deliberate consumption of harmful content. Self-harm is not a failed suicide attempt and it is not attention-seeking; it is most often a regulation strategy, and its presence is one of the strongest single predictors of later suicide risk. It requires direct, unshocked questioning and a proper assessment.

7. The ordinary distress of a normal task. Most referrals are this. A fifteen-year-old who is miserable for three weeks after a breakup, who sleeps badly before exams, who is angry about a move, is doing developmental work. The distinction is duration, impairment, and severity — and the error is not to dismiss ordinary distress, which is real, but to treat

it as a disorder, which pathologises a competent person at the moment she is learning to cope.

The rule that governs all seven. A family therapist screens and refers; she does not diagnose in the therapy room, and she does not manage a psychosis or a developing eating disorder in a family session. The screening obligation is real and the scope limit is real, and holding both is the professional position this week is teaching.

8.2b Self-harm and suicide: what to ask and what to do with it

This section is written to be used. It is deliberately concrete, because the failure mode in this age band is not a wrong intervention; it is a question that was never asked.

The four questions, in this order.

1. "Some young people, when things are hard, hurt themselves on purpose. Have you ever done that?"
2. "Have you ever thought that you would be better off dead, or wished you could go to sleep and not wake up?"
3. "Have you thought about how you would do it?" — the presence of a method, and its availability, is the single strongest predictor of risk.
4. "What has kept you going so far?" — the protective factors, and the beginning of the safety plan.

Why the wording matters. "Are you having any thoughts of self-harm?" is a professional's sentence and gets a professional's answer, which is "no". "Have you ever hurt yourself on purpose?" is a person's sentence and gets an answer. The questions must be asked plainly, without a preamble that signals alarm, and without the therapist's face changing, because the adolescent is watching the face for the verdict on whether she has just made a terrible mistake.

The myth, stated plainly. Asking about suicide does not increase risk. The evidence on this is consistent across settings and the belief that it does is the main reason clinicians do not ask. Say this to parents, in these words, because they will be frightened by the questions and will otherwise interpret them as an intervention that caused the problem.

What to do with a positive answer. In order: do not leave the adolescent alone if risk is current; involve the parents in the same session, with the adolescent's knowledge and, where possible, her agreement about how it is said; remove or restrict means — medication, blades, pesticides, firearms, and anything else available in the household; arrange a same-week or same-day referral according to local protocol; document exactly what was asked, what was answered, and what was done. Then arrange the next contact before the family leaves the building.

The safety plan, which is not a contract. A "no-harm contract" has no evidence behind it and can shift responsibility onto the adolescent. A safety plan is different: it is a written list of (a) the warning signs, (b) the things the adolescent will do first, (c) the people she will contact, in order, and (d) the means that have been removed from her reach. It is

written with her, in her words, and the parents have a copy that says what they will do, not what she has promised.

The family's part. Two things a family can do that matter more than any conversation: reduce access to means, and increase supervision without surveillance. The distinction is the same as Week 7's: a parent who sits in the room is supervision; a parent who reads the diary is surveillance. The second produces concealment, which is the one outcome a safety plan cannot survive.

8.3 The method: a risk assessment a family therapist can actually do

Ten steps, thirty to forty minutes, doable in a family session with the adolescent seen alone for the middle portion. The purpose is not diagnosis. It is to answer three questions: how serious, what else, and what happens next.

Step 1 — Establish the frame first. "I am going to ask you some direct questions. Some of them will feel strange. I ask everybody. If I think you are in danger I will have to act on it, and I will tell you before I do." The last clause is what makes the questions answerable.

Step 2 — Ask about the presenting behaviour in behavioural terms. Not "why did you cut yourself?" but "where, when, how many times, what were you feeling just before, and what happened afterwards?" The afterwards question identifies the function with surprising reliability: relief points to regulation; numbness points to dissociation; punishment points to self-directed anger.

Step 3 — Ask the four questions from 8.2b. In order, plainly, without a preamble.

Step 4 — Map the combination. Sleep, appetite, school attendance, withdrawal, substances, eating, mood, anxiety. The question "what else is going on?" is asked after the disclosure, never before, because before it signals that the therapist has already decided what the problem is.

Step 5 — Ask about the environment, not just the individual. Who knows? Who would she tell? Who in the household is available at eleven at night? What is in the house that could be used? Where does the phone sleep? Is there anyone at school?

Step 6 — Establish the protective factors. What has kept her going; what she is looking forward to, however small; who she would not want to hurt; what she is good at. These are not consolation prizes. They are the material of the safety plan and the best available predictors of who comes back.

Step 7 — Bring the parents in, with the adolescent's knowledge. How it is said is negotiated first: "your mother needs to know about the cutting. Do you want to tell her, or shall I?" The choice matters more than the content, because it preserves the adolescent's standing in her own disclosure.

Step 8 — Restrict means, concretely. Ask what is in the house: medications, blades, rat poison, pesticides, a car, a rope in the yard. Then assign the task of removal to a named adult, with a date. A safety plan that does not change the household's physical environment is a conversation.

Step 9 — Decide the level of care and say it out loud. Outpatient with weekly contact; urgent referral; emergency. Say which, and say why, in front of the family, so that the decision is not a mystery. Families who are told the reasoning cooperate with the plan; families who are handed a plan comply with it until the first difficulty.

Step 10 — Write it down and book the next contact. A one-page safety plan in the adolescent's words, a copy to the parents, a date in the diary before anyone leaves the room. Then document all of it.

8.3a Digital life: the history that is worth taking

Five questions, ten minutes, and they replace the argument about hours.

Question 1 — Where does it sleep? The answer predicts sleep loss more reliably than any measure of use. An adolescent whose phone is in the bed has a device that interrupts sleep, and sleep loss is causally implicated in mood deterioration, which makes this the highest-yield structural question in the week.

Question 2 — What is it for? Connection (the group), escape (the scroll), status (the post), soothing (the same video forty times), or harm (the group that excludes, the person who threatens). Ask for a description of the last hour before sleep, which is when the function is most visible.

Question 3 — Who is in the group, and who is not? Exclusion in this age band is frequently technical: not added, muted, removed. Ask whether she is in the class group and whether she wants to be. The answer is often the whole of the presenting problem.

Question 4 — What has arrived that you did not want? This is the question that produces disclosures of bullying, sexual content, and strangers, and it is asked without the parents in the room. Then: "did you tell anyone? what did they do?" The second question audits the adult system, not the adolescent.

Question 5 — What would you do if you lost it for a week? The answer describes the phone's function better than any inventory. An adolescent who is horrified is describing a social lifeline. An adolescent who is indifferent is describing an escape that has stopped working.

What to do, in order of evidence. Protect sleep (device out of the bedroom, for everyone); protect the one relationship that matters (a named adult who is available and who does not confiscate); build one non-screen domain of competence (this is the "substitute" the stub refers to, and it is always an activity with other people in it); and then, only then, negotiate limits. Limits imposed before the substitute exists produce concealment.

8.4 Three techniques

Technique 1 — The function grid. With the adolescent, place each risk behaviour on a two-axis grid: what it does for me (the five functions) and what it costs me (health, money, school, relationships, freedom). Fill it in her language. The grid does more than a lecture because it is her data, and because it makes visible the behaviours that are cheap and the ones that are expensive — which is not always where the family's anxiety is concentrated.

Technique 2 — The availability map. Draw the household and the route to school, and mark where each substance or means is available: the junction, the flat upstairs, the older cousin, the chemist that sells without a prescription, the drawer in the kitchen. The map converts a moral question into a logistical one, and logistics can be changed. It also routinely reveals that the family has more control over access than it believed.

Technique 3 — The one adult at eleven at night. Ask the adolescent to name the person she would contact at eleven at night if something had happened. If the answer is "nobody", that is the intervention: find, in the household or the extended family, one adult who will agree to be reachable at that hour without asking questions first. This is the cheapest suicide-prevention intervention available to a family therapist and it is almost never written down.

8.5 The first session with a self-harming adolescent and two parents

Phase 1 — The parents' two minutes each (5 minutes). What they found, when, and what they did. The therapist listens for the sequence: discovery, reaction, and what has happened since. The reaction is usually a confiscation, a shouting, or a silence, and each of the three has a predictable consequence.

Phase 2 — The adolescent alone (25 minutes). The frame, then the ten-step assessment, compressed. The four questions are asked in this phase and never in front of the parents, because the parents' faces make the answers impossible.

Phase 3 — Negotiate the disclosure (5 minutes). What will be told, in what words, and by whom. The adolescent is given the first option to tell it herself.

Phase 4 — The joint session (20 minutes). The disclosure; the parents' reaction, which the therapist manages by slowing it down rather than by correcting it; then the means restriction, assigned by name; then the safety plan, written.

Phase 5 — The plan and the next appointment (5 minutes). Level of care, next contact, who telephones whom, and what the parents do tonight. Nothing is left as "we will see".

Why the parents are not in the room for phase 2. Not to protect the adolescent from them, but to protect the information. A sixteen-year-old will not describe the function of her cutting in front of a mother who is crying, and the function is the treatment target. The parents are brought in as soon as the information exists, and they are brought in by design rather than by accident.

8.5a The crisis hour: when the risk is immediate

Most of this week concerns assessment. This section concerns the one hour in which assessment is not the task, because the adolescent in front of you is at immediate risk and the decision cannot wait for a referral.

The four indicators of immediacy. A stated intent, rather than an idea. A plan with a time. Access to the means, in the room or in the house, tonight. And the absence of any named reason to stay alive in the next twenty-four hours. Two of the four is urgent. Three is an emergency, and the therapist's job changes from treating to transporting.

The first move is physical, not conversational. Do not leave the adolescent alone. If she is in the clinic, stay in the room and ask a colleague to attend. If she is at home on the telephone, ask her to go and sit with a named person in the house, now, while you stay on the line. The single most useful thing in the first five minutes is another human being in the same room as her.

The second move is to remove the means, by name. "Where is it? Is it in the house? Who can take it now?" Ask the adolescent first, then the parent. A blade, a bottle of pesticide, a stock of tablets, a car key. The instruction is specific: not "keep her safe" but "take the bottle to the neighbour's house tonight and tell me when it is done". Then record what was said and who confirmed it.

The third move is to tell the parents, with the adolescent's knowledge, in her hearing. "I am going to tell your mother that you have a plan and a time, because she is the person who can be with you tonight. I am not telling her because you did something wrong." This is the one situation in which the therapist does not offer a choice about disclosure, and saying so plainly is more respectful than pretending the choice exists.

The fourth move is the handover, and it has a name in it. Not "go to the hospital". A named facility, a named person if possible, a written note, and confirmation that the family has arrived. A therapist who sends a family to "casualty" has sent them to a building, and a family that arrives at a building at midnight without a name and without a person to ask for will go home again, and will wait until Monday, and the hour will have been lost in the interval.

The fifth move is the follow-up call. Within twenty-four hours, to the family and, with permission, to the adolescent. The call is not clinical; it is the demonstration that the crisis did not end the relationship. Adolescents who are admitted after an attempt and who receive no contact from the referring clinician are markedly less likely to return afterwards.

What not to do. Do not extract a promise. Do not ask the family to "watch her closely" without saying what that means in hours and in people. Do not schedule the next appointment for a fortnight. Do not tell the adolescent that she is being dramatic, and do not tell her that everything will be fine — both are statements she has heard before from adults who then did nothing.

Afterwards, the therapist's own state. A crisis hour is frightening, and the two common reactions are over-reaction (escalating every subsequent disclosure to the emergency pathway, which teaches the adolescent to stop disclosing) and under-reaction (recording the risk as lower than it was, in order to feel that the hour was manageable). Both are corrected in supervision, and the requirement to take a risk assessment to supervision within the week is not bureaucratic. It is the one reliable protection available to a lone clinician against the second error, which is the more dangerous of the two and the harder one to see from inside a hard week.

8.6 Case: the boy whose teacher saw his arm

Referral. Sadiq, sixteen, in Senior Secondary 2, was sent to the school counsellor after a teacher noticed scars on his left forearm during a practical. The counsellor telephoned his mother the same day and referred him to the clinic. The mother, Rukayat, 39, a textile trader, arrives with him. She has not told his father.

The household. Rukayat; Sadiq, the eldest of five children (16, 14, 11, 7, 3); Rukayat's mother, 66; and a younger brother of Rukayat's who helps in the shop. Sadiq's father, 52, lives in Kano with a second wife and telephones on Fridays. He pays school fees and has not visited in fourteen months. Rukayat's income is the household's income.

The discovery. Eight scars on the left forearm, five of them recent, all consistent with a blade. Two on the upper thigh, which the mother did not know about and which the therapist found by asking.

The parents' account, in the first five minutes. "He is quiet at home. He does not tell me anything. He used to be first in his class and now he is eleventh. I thought it was the phone." Then, after a pause: "His father will say I have failed."

The adolescent alone. The four questions are asked in order and answered in order.

1. Have you ever hurt yourself on purpose? "Yes." When did it start? "Last year. After my father stopped coming."
2. Have you ever thought you would be better off dead? A long pause. "Sometimes. Not like a plan. I think about not being here."
3. Have you thought about how you would do it? "I have thought about the gutter on the third floor of the school. But I would not. My mother would die." — A method has been considered; a protective factor is named in the same breath. Both are recorded, and both change the plan.
4. What has kept you going? "My sisters. And my mother. And football."

The function. Asked what happens just before: "it is after the phone call on Friday. He talks to my mother and then she is crying and she says nothing to us and I am the man of the house. I go to the bathroom." Asked what happens after: "it is quiet. For maybe ten minutes it is quiet."

The assessment. The function is regulation, with a secondary self-punishment component: the cutting follows a weekly event, it produces a brief quiet, and it is accompanied by a conviction that he should be managing better. The combination map shows: sleep onset after one in the morning, appetite reduced, school performance fallen from first to eleventh, withdrawal from football in the last two months, no substance use, no disordered eating. Duration: fourteen months, with the first scars dating to the month the father's visits stopped.

The risk formulation, written as it would be in a record. Current risk: moderate. Ideation present, intermittent, without intent or plan, with a named protective factor and a named method that has been considered and rejected. Self-harm: current, increasing in frequency over three months. Means: a razor blade kept in the bathroom, and medication

in the kitchen. Protective: mother's availability, four younger siblings, football, a teacher who noticed. Aggravating: paternal absence, financial pressure, the role of "man of the house" assigned to a sixteen-year-old, sleep loss.

The formulation. A sixteen-year-old carrying an adult role he was given and not prepared for, in a household where the father's Friday telephone call is the week's emotional weather, and where nobody has asked him how he is because he is the one who is supposed to be all right. The cutting is a private regulation strategy for a public situation. It is not attention-seeking: nobody was meant to see it, and the teacher seeing it is the crisis of the case rather than its solution.

The intervention, in five parts.

Part 1 — Means restriction, tonight. The blade is removed by the mother in Sadiq's presence, with the therapist having said why. The kitchen medication moves to the mother's room. The head teacher is contacted with Sadiq's knowledge and asked to make the third-floor landing supervised at break. This is unglamorous and it is the part of the plan with the best evidence behind it.

Part 2 — The safety plan, in his words. Warning signs: "Friday night. When she is quiet after the phone." First three things: "call Tunde" (a teammate), "go and kick the ball outside even if it is dark", "play the guitar for ten minutes". People in order: mother, Tunde, the counsellor, the clinic number. Removed means: blade, kitchen medication. Written on one page, photographed by him and by his mother, and put by the clinic in the record.

Part 3 — Move the Friday. The single structural change of the case. The therapist asks Rukayat, with Sadiq present, what happens after the Friday call. She describes it: she cries in the bedroom, then cooks, then says nothing. The agreement: after the call she will sit with Sadiq and the two eldest for ten minutes and tell them what was said, in one sentence, without asking them to look after her. The content does not change. The secrecy does. A sixteen-year-old who knows what was said no longer has to infer it from his mother's face, and the inferring is what was producing the bathroom.

Part 4 — Give the role back. "Man of the house" is a role without a job description, and Sadiq has been doing it badly because it cannot be done. The therapist asks Rukayat to name, in front of him, three things that are his (school, football, his sisters' homework) and two that are not (money, her marriage, whether his father visits). The list is written. The purpose is to make the boundary visible, because an adolescent cannot refuse a role that nobody has named.

Part 5 — One hour a week with an adult who asks nothing. The counsellor at school, or the football coach, or an uncle. The instruction to the adult is the same as the week's technique: no questions about home, no advice, one hour, once a week, doing something. Sadiq chooses football with the coach on Wednesdays.

The referral. Sadiq is referred for a diagnostic assessment, which returns a picture of moderate depressive symptoms without a formal disorder at that point, and for six sessions of individual work alongside the family sessions. The therapist does not do the

individual work herself; she keeps the family work, because a sixteen-year-old who is self-harming needs two clinicians less than he needs two distinct relationships that are not the same relationship.

8.6a Eight weeks later

The cutting has stopped for six weeks, which is a fact and not a cure. It has been replaced, in Sadiq's own description, by "the guitar and calling Tunde", and he says this with the flatness of someone reporting a task rather than a change.

The Friday conversation happens. Rukayat reports that it is "short and not nice", which is the realistic outcome: the content of the marriage has not changed and is not going to change in this therapy. What has changed is that Sadiq no longer receives it as atmosphere.

The grades have not recovered. The therapist records this as the case's honest limitation: a sixteen-year-old who has lost fourteen months of study does not recover them in eight weeks, and the school has agreed to a reduced subject load for one term.

The most significant change is not in the record. Rukayat has begun telling her brother, who helps in the shop, one true thing a week about the household. The therapist did not plan this. It is the ordinary consequence of a family in which information has started to move, and it is the best available sign that the change will survive the therapy.

What would have changed the plan. If the ideation had included intent, a plan, or a recent preparation, the case would have gone to emergency assessment the same day and the therapy room would have become a holding place rather than a treatment. If the mother had refused to remove the blade, the therapist would have documented the refusal and escalated within the safeguarding framework, because means restriction is not negotiable. If the father had been reachable and willing, one session with him on the telephone would have been attempted, with Sadiq's agreement — and if Sadiq had refused, the refusal would have been respected and recorded.

8.7 Transcript: asking the four questions

Twelve turns, compressed from twenty-five minutes. The transcript's purpose is to show the tone: flat, direct, unhurried, and without a single reassuring phrase in the wrong place.

> **T1 Therapist:** I am going to ask you some direct questions. Some of them will feel strange. I ask everybody I see, and I am not going to be shocked by the answers. If I think you are in danger I will have to do something about it, and I will tell you before I do. All right? — The frame, in four sentences, before anything else. **T2 Sadiq:** Okay. **T3 Therapist:** Some young people, when things are hard, hurt themselves on purpose. Have you ever done that? — Question 1, with no preamble and no euphemism. **T4 Sadiq:** (pause) Yes. **T5 Therapist:** Where? **T6 Sadiq:** My arm. The inside. **T7 Therapist:** How many times? **T8 Sadiq:** I don't know. Many. **T9 Therapist:** (no change of expression, pen down) Have you ever thought that you would be better off dead, or wished you could go to sleep and not wake up? — Question 2, asked in the established tone. The pen going down is deliberate: writing while a person answers this question makes them a specimen. **T10 Sadiq:** (long pause) Sometimes. **T11 Therapist:** Have you thought about how you would do it? **T12 Sadiq:** (longer pause) The gutter. At school. The third floor. — The answer that changes the plan: a method, a place, and a school. Everything after this turn is logistics, and the therapist's next sentence is not a question about feelings.

8.8 Laboratory: the risk assessment, marked

Aim. To complete the ten-step assessment with a reticent adolescent in thirty-five minutes, obtain the disclosure, and leave with a signed safety plan and a documented level of care.

Procedure. Trios: therapist, adolescent (briefed with a disclosure that includes a method, a protective factor, and one detail she will withhold unless asked twice), observer. Thirty-five minutes, then ten minutes of feedback against the rubric. The withholding is the exercise's centre: the briefed adolescent will not volunteer the second location of the scars, and a therapist who does not ask about the body directly will miss it.

Marking rubric (25 marks).

Criterion	0–1	2–3	4–5
The frame	Not stated; or stated as a warning	Stated, but after the first question	Stated first, including the limits, and the adolescent's assent obtained
The four questions	Not asked, or asked with a euphemism	Asked, but in a preamble-laden way	Asked plainly, in order, without a change of tone or expression
Function	Not established	Asked what happened before	Asked before and after, and the function named in the adolescent's words
Combination	Only the presenting behaviour	Two or three domains covered	All seven domains screened, including sleep and eating
Means	Not discussed	Discussed generally	Specific inventory, named adult, date, and confirmation in the room
The plan	Advice	Written plan without protective factors	Written in the adolescent's words, with warning signs, sequence, means removed, and a booked next contact

The distinction that carries most of the marks. The top band requires the student to ask question 3 (the method) after a positive answer to question 2, without softening. Most trainees stop at "sometimes I think about not being here", because the disclosure is distressing and because stopping feels kind. It is the moment at which the assessment is either completed or abandoned.

8.9 Six errors

Error 1 — Not asking. The largest error by volume, and the one with the clearest cause: the fear of causing harm. The evidence is unambiguous that asking does not increase risk. A therapist who works with adolescents and does not ask is not protecting them; she is protecting herself.

Error 2 — Asking in a way that can only be answered "no". "You are not thinking of doing anything silly, are you?" is a question with the answer built into its grammar. Every one of the four questions is closed, direct, and free of the answer.

Error 3 — Promising confidentiality you cannot keep. An adolescent who discloses self-harm after being told "everything stays in this room" has been misled at the moment of maximum vulnerability, and the breach of that promise is often the end of treatment. State the limits first, every time.

Error 4 — Making the family the enemy. Parents who react with confiscation, shouting, or silence are frightened, not cruel. A therapist who treats them as the problem loses the household, and the household is the safety plan.

Error 5 — The no-harm contract. A signed promise not to self-harm has no evidence behind it and shifts responsibility onto the adolescent at the moment she has least capacity to carry it. Replace it with a safety plan that describes what she will do, who she will call, and what has been removed from her reach.

Error 6 — Treating the disclosure as the endpoint. The disclosure is the beginning of the assessment. The commonest failure in supervision is a therapist who reports, with relief, "he told me everything", and who cannot say what the level of care was, what was removed from the house, or when the next contact is.

8.10 Culture, faith and adolescent risk

The household's reputation. In many settings an adolescent's self-harm or substance use is experienced by the family as a judgement on the household, and the first clinical obstacle is not the behaviour but the family's need for it not to be known. A therapist who telephones the school, calls a meeting, or writes to a relative without an explicit agreement will lose the family. The sequence is always: agree who knows, then act.

Faith, deliverance and the clinic. Families may hold that the behaviour is spiritual, and may pursue prayer, fasting or deliverance alongside or instead of clinical care. The workable position is neither collusion nor confrontation: accept the household's framework as the language of concern, ask what the prayer or the fasting is for, and negotiate one non-negotiable — that the safety plan is in place while the spiritual work

continues. A family that is told to choose between its faith and the clinic will choose its faith, and the adolescent will be the one who loses.

Gender and the boy who must not be weak. Sadiq's case is a gender case before it is anything else. A sixteen-year-old boy in a household where he is "the man of the house" has been given an emotional prohibition along with the role. The clinical move is not to tell him to express his feelings; it is to give him a permitted domain — football, an uncle, a coach — in which being looked after is not a humiliation.

The second wife, the absent father, and the therapist's judgement. Polygynous arrangements, labour migration, and long paternal absences are ordinary structures in many of the households this book is written for. A therapist whose formulation requires the family to become a different kind of family will produce a plan that nobody follows. The question is always what can be changed inside the arrangement that exists — the Friday conversation, the role list, the uncle's hour — rather than what would have to change for the problem to disappear.

> **DEBATE**
> "A sixteen-year-old's self-harm should be disclosed to the parents whether or not she agrees." Argue for: the parents are the safety plan, the risk of concealment is death, and the adolescent's judgement is the thing that is compromised. Argue against: disclosure against her will ends the therapeutic relationship at the moment it is most needed, the adolescent will conceal more effectively next time, and a competent sixteen-year-old has a legitimate interest in how her own body is discussed. Then write the rule you would follow, including the case in which she refuses and the risk is moderate rather than imminent.

8.11 Evidence note

What the research supports. Means restriction is the intervention with the strongest evidence in suicide prevention at population level: reducing access to a common method reduces deaths, including in adolescents, and the effect is not merely substitution of method. Safety planning, as a structured and collaborative written intervention, has better support than no-harm contracts, which have essentially none. Sleep disturbance is a prospective predictor of adolescent mood deterioration, which is why the question about where the phone sleeps is a clinical question and not a parenting preference.

What is weaker. The evidence for digital-detox interventions is thin and the field is moving faster than the trials. The most defensible claims are structural (sleep protection, one non-screen domain of competence, one available adult) rather than quantitative (a number of hours). The evidence on family-based treatment for adolescent self-harm is promising but based on small trials with heterogeneous comparators.

What is not supported. That asking about suicide increases risk — the evidence is consistent that it does not, in clinical and school settings. That self-harm is attention-seeking — the clinical literature describes a predominantly private behaviour whose discovery is usually experienced as a crisis. And that confiscation of devices

reduces harm — the observable effect is migration to unmonitored devices, which reduces the parents' knowledge without reducing the exposure.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same paternal absence produces, in one sixteen-year-old, a determination to succeed and, in another, fourteen months of silent cutting. What differs is not the input but the presence of one adult who asks, and that is what the therapist is trying to install.

8.12 Key terms

Function — what a risk behaviour does for the person; the first question of the assessment.

Combination risk — the multiplicative effect of several concurrent adversities, as against the effect of any one.

Means restriction — reducing access to the method; the best-evidenced single intervention in suicide prevention.

Safety plan — a written, collaborative list of warning signs, coping steps, contacts and removed means.

No-harm contract — a signed promise not to self-harm; not evidence-based and not a substitute for a safety plan.

Prodrome — the period of subtle change preceding the frank onset of a disorder, particularly psychosis.

Duration, impairment, severity — the three dimensions that distinguish disorder from distress.

Digital function — what a device is doing in a person's life: connection, escape, status, soothing, or harm.

Protective factor — a circumstance that reduces risk; in this age band usually a named adult and a domain of competence.

Level of care — the decided setting and intensity of treatment: outpatient, urgent, or emergency.

8.13 Self-test

1. State the four questions of the risk assessment, in order, and say why question 3 follows question 2.
2. A parent asks whether asking about suicide will put the idea in her daughter's head. What do you say, and what is the evidence?
3. Name the five functions of adolescent risk behaviour and give one intervention matched to each.
4. Why is means restriction the best-evidenced intervention in this field, and what does it require in practice?

5. Distinguish a safety plan from a no-harm contract, and say why the difference matters clinically.
6. Name the three dimensions that separate disorder from distress, and apply them to a fifteen-year-old who has been miserable for three weeks after a breakup.
7. In the case of Sadiq, what was the single structural change, and what did it change?
8. What is the "one adult at eleven at night", and why is it described as the cheapest intervention available?

Answers. (1) Self-harm; suicidal ideation; method and intent; protective factors. Question 3 follows because a positive answer to question 2 identifies ideation while question 3 identifies risk, and the method is the strongest single predictor. (2) That it does not — the evidence across clinical and school settings is consistent, and the belief that it does is the main reason clinicians fail to ask; say it plainly, because the parent's fear will otherwise be interpreted as a reason for silence. (3) Belonging — change the group or build an alternative group; escape — address what is being escaped, usually the household's evening; status — build a domain of real competence; self-soothing — teach a regulation skill and protect sleep; self-punishment — address the self-directed anger, usually with individual work. (4) Because it does not require the person in crisis to make a decision at the moment of crisis; in practice it requires a specific inventory of the household, a named adult, and a date. (5) A safety plan is collaborative and describes what the adolescent will do and what has been removed; a no-harm contract is a promise that shifts responsibility onto her. The first builds capacity, the second borrows it. (6) Duration, impairment, severity — three weeks after a breakup with no impairment in school or friendships and moderate severity is ordinary distress and should be respected as such rather than treated. (7) The Friday conversation: naming what was said, to the two eldest, in one sentence, without asking them to manage her feelings — it converted an atmosphere that had to be inferred into information that could be held. (8) A named adult reachable at that hour without interrogation first; cheapest because it requires no clinical resource, and effective because the hours after ten at night are when risk is highest and when the adolescent is least able to reach a professional.

8.14 Reflective task

Write down the last time you asked someone a direct question about their safety, or the last time you avoided one. If you avoided it, reconstruct the sentence you used instead, word for word. Then answer: what were you protecting? Most trainees find that the substitute sentence protected the therapist's comfort, and that the discomfort was located in a single word which, once said aloud in a rehearsal, is never as difficult again.

8.15 Further reading

- Hawton, K., Saunders, K. and O'Connor, R. "Self-harm and suicide in adolescents" — the epidemiology and the evidence on means restriction.
- Stanley, B. and Brown, G. "Safety planning intervention" — the structured alternative to no-harm contracts.

- Steinberg, L. "Risk taking in adolescence" — the peer-amplification evidence behind this week's first concept.
- Nock, M. "Why do adolescents hurt themselves?" — the functional account used in the case.
- Odgers, C. and Jensen, M. "Annual research review: adolescent mental health in the digital age" — the most careful summary of a thin evidence base.

Next: Week 9 — Adult development: work, intimacy and the middle years.

Week 9 — Adult development: work, intimacy and the middle years

Developmental tasks. Intimacy versus isolation (Erikson, roughly the twenties and thirties); work and career identity, including the difference between a job, a career and a calling; generativity versus stagnation (the forties); and the "social clock" — the culturally shared timetable for marriage, children, promotion and retirement, and the distress that follows when a life runs off it.

Why this matters clinically. Most couple problems presented in clinics are adult developmental problems wearing a relational mask: a thirty-two-year-old's struggle with intimacy, a forty-five-year-old's reckoning with a career, a fifty-eight-year-old's grief for the life they did not have. The therapist who hears only the relationship will treat the mask. The therapist who hears the developmental task will treat the person, and the relationship will follow.

Life structure and the "seasons" of adulthood. Levinson's seasons, Vaillant's longitudinal cohorts, and the midlife literature are best treated as metaphors with empirical support, not as laws. The strongest finding in the whole of adult developmental research is **continuity with change**: people change, but in patterned ways that reflect early relationships and current context. The man who was defended at twenty-five is defended at fifty, in a different vocabulary, about different things.

What this week does not claim. It does not claim that there is a crisis everyone has at forty, that marriage has stages everyone passes through in order, or that a person who has not achieved generativity by fifty is damaged. The evidence supports tasks, timetables, and patterns. It does not support prescription, and the therapist who turns a developmental map into a standard of normal adulthood will spend the week hurting families who are simply living on a different schedule.

> **CASE**
> A forty-five-year-old man presents with "irritability". Developmental enquiry finds that he has been passed over for a promotion he had organised his entire adult life around, that his father died eighteen months ago, that his eldest child has left for university, and that he has begun to notice that he has no friends he did not acquire through work. The irritability is at the surface. Under it are four adult developmental events arriving in the same eighteen months.

9.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The mask	Why adult developmental problems arrive as relationship complaints	Take a couple complaint and name the developmental task inside it
8–22	Intimacy versus isolation	What intimacy actually requires, and the four adult forms of not doing it	Score themselves on a five-item intimacy audit, privately
22–36	Work identity	Job, career, calling; and the difference between a bad month and a bad decade	Classify six work presentations
36–48	Generativity	The three routes — children, work, mentoring — and stagnation as its absence	Name three people who were generative towards them
48–62	The social clock	On-time and off-time events, and the distress of the off-time life	Place four life events on the clock and mark the culturally expected age
62–74	The midlife reckoning	How it presents in a clinic, and the two ways therapists pathologise it	Rewrite two diagnostic-sounding formulations as developmental ones
74–84	Continuity with change	What longitudinal studies actually find, and what they do not	Predict a fifty-year-old from a twenty-five-year-old in a case vignette
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Adults present with a symptom and a story, and the story is usually chronological but not developmental: it lists events and omits tasks. The lecture's central demonstration is to take one ordinary adult complaint and place it against the developmental timetable, so that the class sees the difference between what happened and what it cost developmentally. Once students can do that, they stop treating forty-five-year-olds as thirty-year-olds with more money.

The sentence the lecture must land. "By forty-five most people are not struggling with their marriage; they are struggling with a question their marriage has been asked to answer, and the marriage is not equipped to answer it."

9.2 Five concepts that organise the week

1. Intimacy versus isolation. Intimacy is not affection and it is not sex. It is the capacity to be known by another person without managing the version of oneself that is known — which requires, first, that there is a self to know. This is why Erikson placed identity before intimacy, and why the sequence matters clinically: a couple in which one partner has never settled who they are will present with "communication problems" that no communication exercise will touch.

The four adult forms of not doing intimacy are worth memorising, because they present as different referrals.

- **Fusion** — the person cannot tolerate separateness; difference is experienced as abandonment, and the couple's argument is always about closeness.

- **Pseudo-intimacy** — the person performs closeness; the couple is highly affectionate in public and has no private conversation.
- **Isolation** — the person keeps a whole domain of life outside the relationship; the complaint is "he does not let me in".
- **Serial replacement** — the person changes partners rather than being known by one; the pattern is visible across relationships and invisible within any one of them.

2. Work identity: job, career, calling. A job is what you do for money. A career is a trajectory with advancement. A calling is what you would do anyway. The three are not ranked; a person with a job and a rich life outside it is not deficient. The clinical significance is in the mismatch: a person who has organised their identity around a career and loses the trajectory has lost more than income, and a person who experiences their work as a calling and finds it meaningless is in a particular kind of distress that will not respond to work-life balance advice.

3. Generativity versus stagnation. Generativity is the concern for establishing and guiding the next generation, and it has three routes: one's own children, one's work, and mentoring. The clinical value of the concept is that it provides an alternative to the two things adults in distress usually believe are their only options — more achievement, or resignation. A forty-eight-year-old man who is stagnating at work may be generative as a mentor, as a son, as an uncle, as a coach, and the therapeutic move is to find where the generative capacity can go rather than to fix the job.

4. The social clock. Every culture has a timetable: the age at which one is expected to marry, to have a first child, to be established, to be a grandparent, to retire. The clock is not usually stated, which is what makes it powerful. Distress follows off-time events far more than expected ones — a first marriage at forty-five, a first child at nineteen, a promotion at twenty-nine, a retirement at fifty-five, a bereavement at thirty. The clinical implication is that the therapist must ask not only "what happened?" but "when, and where were you supposed to be by then?"

5. Continuity with change. Longitudinal evidence supports the view that adult personality is stable in its structure and changing in its expression: the defended young adult becomes the defended middle-aged adult, with better manners and worse consequences. The clinical use is a caution against two errors: the error of expecting a person to become someone else in therapy ("he will never be warm"), and the error of assuming nothing can change ("this is just who he is at forty-five"). What changes is usually the relationship to the pattern, and the practical arrangements around it.

9.2a The social clock and the off-time life

The concept is simple and the clinical consequences are large. Five off-time situations account for most of the developmental distress seen in adult clinics.

Off-time marriage and the unmarried thirty-five-year-old. In cultures with a strong marital timetable, an unmarried adult in their mid-thirties is subject to continuous enquiry, and the enquiry is experienced as an assessment of worth. The clinical presentations are anxiety, depression, or a relationship entered under pressure and

presented as a decision. The therapist's task is to separate the person's own desire from the timetable's, which is done with one question: "if nobody ever asked you again, what would you want?"

Off-time parenthood, in both directions. A first child at nineteen and a first child at forty-two produce different but comparable difficulties: in the first, the parent's own development is interrupted; in the second, the parent's peers are launching adolescents while they are managing toddlers, and the grandparent generation may be unavailable or dead.

The early promotion and the plateau. Being established "too early" produces a lifelong problem of comparison; being passed over at forty-five produces the case in this week's stub. Both are versions of the same injury: the timetable said one thing and the life delivered another, and the gap is experienced as personal failure rather than as a fact about an organisation.

Off-time bereavement. Losing a parent at twenty-five is statistically ordinary and developmentally early; losing a parent at seventy is developmentally "on time" and contains far less of the "I was not ready" quality. The distinction matters because the same event produces different clinical pictures depending on its position on the clock.

Off-time retirement. Retirement at fifty-five in a culture where sixty-five is expected produces a particular shame, especially for men whose identity is occupational. The clinical picture is often depression misdiagnosed as a physical complaint, and the question that opens it is "what were you supposed to be doing now?"

How to use the clock without imposing it. Ask about the timetable, do not supply one. "In your family, when were people supposed to marry? When were they supposed to be settled? Where are you on that?" Then listen for the gap. A therapist who tells a client that the timetable is oppressive — however true — will be heard as dismissing the family, and the client will defend a clock she has just spent an hour telling you she resents.

9.2b The four adult tasks as they present in couple therapy

Adults do not present their developmental tasks; they present their partners. The translation table below is the week's practical core, because it converts a couple complaint into a developmental hypothesis in one step.

The complaint	The developmental task underneath	The question that tests it
"He never talks to me"	Intimacy — the capacity to be known, or the absence of a self to offer	"What would he have to say that would frighten him?"
"She is never satisfied"	Work identity — a calling unfulfilled, or a career stalled	"Whose standard is she failing to meet?"
"There is no us any more, only the children"	Generativity, collapsed into one route	"Who were you generative towards before the children?"
"He changed after his father died"	Off-time bereavement plus the collapse of a generation	"Who is the eldest in his family now?"

The complaint	The developmental task underneath	The question that tests it
"We have nothing to say now the children have gone"	Launching — the couple's identity was outsourced	"What did you talk about before them?"
"She has become obsessed with her mother's care"	Generativity, reversed; the "responsible child" role	"Who decided she was the responsible one?"

The rule that governs the table. Each cell is a hypothesis, never a finding. The therapist offers it as a question, not a formulation, because a couple told that their problem is developmental will immediately begin to argue about whether it is.

9.3 The method: the adult developmental history

Eight questions, twenty to thirty minutes, taken with the individual present and the partner listening. The order is deliberate: it moves from the concrete to the evaluative, so that the person is describing their life before they are interpreting it.

Question 1 — Where were you at twenty-five, and what did you think your life would be by now? The answer produces the person's own timetable, in their own words, and it is the baseline against which everything else is measured. Write the answer down verbatim; it will be quoted back in session four.

Question 2 — What happened instead? Not "what went wrong". The phrasing allows for both gains and losses, and it prevents the interview from becoming a catalogue of failure in front of a partner who is listening.

Question 3 — What are you doing that you did not choose? This surfaces the inherited life: the family business, the profession chosen to satisfy a parent, the move to a city for a spouse's work, the second child who was not planned. The answer is frequently the first time the partner has heard it.

Question 4 — What are you doing that nobody in your family has done before? The counterweight to question three, and the source of legitimate pride. Adults in distress systematically under-report their own novelty, and the therapist's task is to make it visible in front of a witness.

Question 5 — Who depends on you, and what do they need? Generativity, mapped concretely: children, parents, siblings, staff, a congregation, a community. The answer is often the discovery that the person is highly generative and has no route to receive.

Question 6 — Who did you lose, and when? Deaths, separations, migrations, the end of a friendship, the loss of a country. The timing is asked for because off-time losses carry more weight.

Question 7 — What do you do that is not for anyone else? The question that identifies the person's own territory, which has frequently collapsed in the preceding decade. If the answer is "nothing", that is the finding, and it is often more actionable than anything in the presenting complaint.

Question 8 — What would you want your life to have been about? The integrity question, asked early because adults answer it more honestly at forty-five than at seventy, when it has become a verdict rather than a direction.

How to record the answers. Not as a narrative. As an eight-line grid with the questions as headings and the answers as the person's own words. The grid is given to the person, which is itself an intervention: most adults in a couple clinic have not seen their own life summarised on one page.

9.3a The midlife reckoning: what it is and what it is not

The popular notion of a "midlife crisis" — a sports car, a younger partner, an abandoned career — is largely a marketing story with weak empirical support. What the evidence supports is something narrower and more useful: in the forties and early fifties, adults conduct a reckoning between the life they have built and the life they expected, and the reckoning produces either a reorganisation or a set of symptoms.

The four triggers. A loss of a parent (the collapse of the generation ahead, and the arrival of one's own mortality as a fact rather than an idea). A child's departure (the loss of a daily role). A work event — promotion, plateau, redundancy, or the recognition that the ceiling has been reached. And a physical change — illness, injury, or the ordinary evidence of ageing that arrives earlier for some bodies than others.

How it presents. Not usually as a crisis. As irritability, sleep disturbance, increased drinking, withdrawal from the couple, an obsessive new interest, an affair, a sudden conversion, a resignation letter, or a somatic complaint that no physician can explain. The commonest presentation in a family clinic is irritability with no stated cause, which is why this week's case begins there.

The two ways therapists go wrong. The first is to pathologise: to diagnose depression in a person who is grieving a life, and to medicate or cognitive-restructure a reckoning that needs to be conducted. The second is to romanticise: to treat the affair, the resignation, or the abandoned family as a legitimate expression of midlife need, which converts a developmental event into a justification for harm.

The clinical position between them. Take the reckoning seriously as a developmental task, name it as such, and hold the person accountable for what they do while conducting it. The sentence that does both: "what you are describing is a real question, and it does not entitle you to do whatever answers it."

The couple's part. A midlife reckoning is conducted in front of a partner who is having their own. The clinical work is rarely with one person; it is with two people whose timetables have diverged. The question that organises it: "what did each of you expect to be doing at this age, and what has each of you given up to get here?" The answer is usually the first honest accounting the couple has done in a decade.

9.4 Three techniques

Technique 1 — The lifeline, drawn by the person. A horizontal line with the person's age along it, and vertical marks above the line for high points and below for low points, annotated in their own words. Twenty minutes, one sheet of paper, no interpretation while drawing. The technique works for four reasons: it makes the life visible as a shape rather than a series of complaints; it surfaces the pattern of transitions; it produces material the partner has never seen; and it gives the therapist a document to return to. The instruction that makes it work is "put the things that mattered, not the things that should have mattered".

Technique 2 — The two clocks. Two columns on a sheet. Left: "when I expected to…" Right: "when it actually happened". Marriage, children, house, promotion, the death of a parent, retirement. The gap between the columns is the developmental distress, quantified by the person themselves. The technique is particularly effective with couples, because partners routinely discover that they are running on different clocks and have never said so.

Technique 3 — The generativity audit. Three columns: "who I am responsible for", "what they need from me", "what I receive". The third column is almost always empty or filled with the same item repeated. The audit is not a complaint about ingratitude; it is a map of a system with no return path, and the intervention that follows — one hour a week that is not for anyone else — is drawn directly from it.

9.5 The first session with a forty-five-year-old and his wife

Phase 1 — Both, and the complaint in each person's words (15 minutes). The referrer has written "irritability". The therapist asks each partner to describe the last bad week, with one example each, and writes the examples down. This prevents the session from becoming an argument about character.

Phase 2 — The individual history, with the partner listening (25 minutes). The eight questions, asked in order. The partner is asked not to comment, and is told why: "I want you to hear this without having to answer it. You will get your turn." The instruction is usually obeyed, and what follows is the most valuable twenty-five minutes in the case.

Phase 3 — The partner's turn, three questions (10 minutes). What did you hear that you did not know? What did you expect your life to be at this age? What have you given up? The third question is the one that converts the session from an assessment of one person into a developmental conversation between two.

Phase 4 — The shared finding (5 minutes). The therapist names, in one or two sentences, the developmental tasks she heard, and checks them: "I heard four things arriving at once. Is that right?" The couple corrects her, and the corrected version is the formulation.

Phase 5 — One task each (5 minutes). One thing for him, one for her, and one for the pair. Not a programme. Written down, with a review date.

Why the partner listens before she speaks. Because the couple's usual conversation is duelling accounts of the same events, and the developmental history is not an account of

events; it is an account of expectations. A partner who hears her husband's answer to "what did you think your life would be by now?" has received information that no amount of arguing about the dishes could have produced.

9.6 Case: the man who was passed over

Referral. Dapo, 45, a bank branch manager, referred by his general practitioner after two visits with "fatigue and stomach trouble" for which no cause was found. He attends with his wife, Kemi, 43, a pharmacist, who says in the first minute: "he is angry all the time and he will not say why."

The household. Dapo and Kemi, married nineteen years. Tolu, 18, in her first year at a university in another state. Femi, 14. Simi, 9. Dapo's mother, 71, widowed eighteen months ago, living alone in Ibadan in the house Dapo grew up in. Kemi's mother, 68, lives with Kemi's brother in Abuja. The household is comfortable and stretched.

The presenting complaint, in each person's words. Dapo: "I am tired. I do not sleep. I do not enjoy anything." Kemi: "he shouts at the children over nothing. He sits in the car outside the house for twenty minutes before he comes in. Last month he did not come to my sister's wedding and he would not say why."

The eight questions, and what they produced.

1. At twenty-five, what did you think your life would be? "By forty-five I would be a regional director. I would have built the house in Ibadan. My father would have seen it." — Written down verbatim; quoted back in session four.
2. What happened instead? "I got the branch in 2019 and I have been there six years. In March they gave the region to a man who is thirty-eight and who I trained." A pause. "My father died in April last year. I did not build the house."
3. What are you doing that you did not choose? "The branch. I did not want the branch. I wanted the region." Then, after some pressing: "I did not choose to be the one who is responsible for my mother. It was decided. I am the first son."
4. What are you doing that nobody in your family has done? Long pause. "I have never lost a job. Twenty years. My father lost three." — The first item of legitimate pride in the session, and it arrives only because the question was asked.
5. Who depends on you? "Kemi, three children, my mother, my younger brother's school fees, and about forty staff." What do they need? "Everything. I am the roof."
6. Who did you lose, and when? "My father, April last year. And Tolu left in September." — Two losses in the same eighteen months, one a death and one a departure, and both are collapses of a role.
7. What do you do that is not for anyone else? A very long pause. "Nothing. I used to play squash." When did you stop? "When my father was ill."
8. What would you want your life to have been about? "That my children did better than me. And that my father was proud." — The generativity answer and the foreclosure answer arriving in one sentence, which is the formulation of the case.

Kemi's three questions, and what they produced.

What did you hear that you did not know? "That he wanted the region. He never told me. He told me it was a good job." What did you expect your life to be at this age? "I expected to have some time. The children are older. I thought we would have a life again." What have you given up? "My mother lives with my brother. I have never said this to anyone, but I gave up my mother because I was managing his." — The sentence that reorganises the session, and it arrives from a question the therapist asked of the partner rather than of the patient.

The five findings.

1. Four developmental events in eighteen months. A work plateau (the ceiling reached at forty-five, in public, in front of a man he trained); a bereavement (his father, and with him the audience for the life project); a launching (Tolu's departure, which removed the daily evidence of his generativity); and a reversal of generativity (his mother's widowhood, which converted him from the person who receives care to the person who must provide it, permanently).

2. The social clock has failed and nobody has said so. Dapo's timetable said: regional director by forty-five, house built, father present. Every item on it has not happened, and the failure is experienced as personal rather than structural. He has not told his wife because telling her would require him to say the words, and the words are the injury.

3. The presenting symptom is a reckoning conducted badly in private. Sleeping badly, sitting in the car, drinking two beers more than he used to, shouting at a nine-year-old about shoes. None of this is a disorder and all of it is a person conducting a reckoning without a single interlocutor.

4. The marriage has become an administration. Kemi manages the household, Dapo provides; the couple's conversation is logistics and money. This is the ordinary outcome of two careers and three children, and it is also the reason the reckoning cannot be conducted at home: there is no room in the schedule for a conversation that does not have an agenda.

5. The live conflict is about his mother, and it is not about his mother. Dapo wants his mother to move in. Kemi does not. Both positions are legitimate; neither has been stated as a position. Dapo experiences Kemi's resistance as a betrayal of the family; Kemi experiences Dapo's proposal as the final expropriation of a life she has already given two decades to. The therapist's job is to make both statements sayable in the same room, not to resolve them.

The formulation, in one paragraph. A forty-five-year-old man whose life structure has been organised around a single audience — his father — and a single trajectory — the region — has lost both within eighteen months, and has no vocabulary, no interlocutor and no time in which to conduct the resulting reckoning. His irritability is the surface of a grief that has no permitted form. His marriage has become an administration, and the one live decision in front of the couple — whether his mother moves in — is being conducted as a loyalty test rather than as a household negotiation.

The intervention, in five parts.

Part 1 — Name the reckoning. In the second session the therapist says, in front of both: "four things have happened in eighteen months and each of them has taken away a role. That is not an illness. It is a lot, and it has arrived without a single conversation in which you were allowed to say it out loud." Dapo cries, briefly, and is embarrassed. Kemi takes his hand. Nothing else in the session matters as much.

Part 2 — Restore one non-instrumental hour. The generativity audit has produced an empty column. The task is one hour a week that is not for anyone — squash, restored, on Saturday mornings. The therapist is specific about the obstacle: Dapo's objection is that it is self-indulgent, and the answer is that a man who receives nothing has nothing to give, which is a statement about the household's economics rather than about his entitlements.

Part 3 — The mother, as a negotiation with four options. The therapist writes four options on the board, not two: she moves in; she stays and a paid carer is arranged; she moves to a flat nearby with support; she moves in for a trial of three months with a review. The purpose is to break the binary that has made the argument unwinnable. The couple chooses the fourth, and the review date is written into the plan.

Part 4 — The brother's school fees, named. A second loyalty obligation, never mentioned in the referral and discovered by question five. The therapist asks one question: "what was the agreement, and who made it?" The answer reveals an arrangement made by Dapo alone, presented to Kemi as a fact. Renegotiating it is not the point; naming it as a joint decision rather than a unilateral one is, because the pattern — a man deciding alone and then resenting alone — is the marriage's actual problem.

Part 5 — One conversation a week that is not logistics. Twenty minutes, Sunday evening, agenda-free, no children, no money. The rule: neither party may raise a problem in that twenty minutes. The purpose is not intimacy in the romantic sense; it is to rebuild the channel through which a reckoning could be conducted at home rather than in a car.

9.6a Five months later

The mother did not move in permanently. She moved into a flat two streets away with a carer who comes daily, and the arrangement is reviewed every three months. Dapo visits on Wednesdays and Sundays, which is more than he visited when she lived alone in Ibadan.

Squash has become twice a week, with a man Dapo knew at university and had not telephoned in nine years. The therapist did not suggest the friend; the squash did.

The region has not been offered and will not be. In session seven Dapo says, unprompted: "I am not going to be a regional director", and the sentence is flat rather than bitter. Kemi says afterwards, privately, that this was the moment she stopped being afraid of him.

The drinking has reduced without being addressed. The therapist notes this in the record as an example of a symptom that resolves when the function it was serving is met elsewhere, which is the general principle this week has been teaching.

The honest limitations. Dapo has not grieved his father, in any sense a therapist would recognise. He has stopped being angry, which is not the same thing, and the therapist

says so in the final session rather than claiming a resolution. The couple's twenty minutes survives about three weeks in four. And the eldest child, Tolu, has not been part of the work at all, which the therapist records as a gap: a launching that happens to a family in difficulty usually needs the person who left to be spoken about, at least once.

What would have changed the plan. If Dapo had described persistent low mood with anhedonia for more than two months and impairment at work, the plan would have begun with a diagnostic assessment and possibly medication, alongside the developmental work. If the drinking had reached a level with withdrawal symptoms, substance work would have taken precedence. If Kemi had refused to continue after the mother's proposal, the work would have become a couple's negotiation about competing obligations to two sets of parents, which is a different treatment with a different endpoint. And if Dapo had disclosed an affair — which the therapist asked about directly, and which he denied — the reckoning would have been conducted against a background of breach, and the sequence would have been different.

9.7 Transcript: the lifeline, drawn in front of the couple

Twelve turns from the second session. The therapist has one sheet of A4 and a pen, and she has given the pen to Dapo.

> **T1 Therapist:** I want to draw your life on this page. A line across the middle, and your age along it. Above the line, the things that were good. Below, the things that were hard. Not the things that should have mattered — the things that did. — The instruction that makes the technique work, and the only instruction given. **T2 Dapo:** (draws, silent, for about four minutes) **T3 Therapist:** (says nothing) — Four minutes of silence while a man draws his life. The impulse to comment at the first mark is strong and must be resisted; the drawing is the intervention and the therapist's commentary would take it back. **T4 Dapo:** (stops) There. (handing it over) **T5 Therapist:** (reading) 1998, graduation. 2001, marriage. 2003, Tolu. 2007, the first branch. 2011, your mother's illness. 2014, Femi. 2019, this branch. (pause) 2024, your father. (pause) What is that mark in 2016? — The therapist reads the marks back flatly, without evaluation, and then asks about one unexplained mark rather than about the obvious ones. The unexplained mark is where the person has put something they have not decided how to describe. **T6 Dapo:** That is when I stopped the squash. (pause) My father was ill from 2015. **T7 Therapist:** So the squash went first. **T8 Dapo:** (quietly) Yes. **T9 Kemi:** I did not know that. I thought you were tired. — The partner's first contribution, and it is not an argument. This is the effect the technique is designed to produce: a fact the partner did not have, delivered without accusation. **T10 Therapist:** (to Kemi) What do you notice about the line? — The question is given to the partner deliberately. It converts her from an audience into a reader, which is the difference between a session about a man and a session about a couple. **T11 Kemi:** Everything above the line is work or children. There is nothing on it that is just him. — She has found the generativity audit without being shown it. **T12 Therapist:** (to both, quietly) That is what I noticed too. (pause) Dapo, what would you put on it if you had an hour a week that was not for anybody? — The question is asked of him, not proposed by the therapist, and it is asked last, when the evidence for it is on the page in his own hand.

9.7a What to do when the partner weeps and the client does not

A practical note that belongs to this week and recurs in every adult case.

In the session above, Kemi weeps twice and Dapo does not. Trainees read this as evidence that Kemi is "more in touch" and Dapo is "defended", and they begin to work on Dapo's defences. The reading is usually wrong, and the intervention that follows from it usually fails.

Three alternative explanations, and how to tell them apart. First, timing: the partner may have had years to grieve the marriage in private and is now crying about something old, while the client is hearing his own life described aloud for the first time and is in the numb phase that precedes grief. Second, permission: in many households the man has never cried in front of anyone and the prohibition is stronger than the feeling. Third, role: one partner is the expressive one and the other the containing one, and the arrangement is stable rather than pathological.

The test. Ask each of them, separately and plainly: "when did you last cry, and who was there?" The answers distinguish the three explanations within a minute. A man who cries alone in a car is not defended; he is unaccompanied.

What to do in the session. Do not pursue the non-crying partner's tears. Attend to the one who is crying, briefly and without fuss — a sentence, a pause, a glass of water — and then return to the material. The non-crying partner is watching how the therapist treats the crying one, and what he learns in those thirty seconds determines whether he will ever cry in this room.

What to say afterwards, if anything. One sentence at the end of the session, addressed to him and not to the couple: "you did not have to do anything while she was crying, and you stayed." Naming the staying is more useful than naming the absence of tears, and it is accurate.

9.8 Laboratory: the developmental history with a partner present

Aim. To take the eight-question history with an individual whose partner is in the room, without losing the individual to the couple.

Procedure. Trios: therapist, client, partner. The trainer briefs the client with a life containing three off-time events and one foreclosed choice; the partner is briefed to interrupt twice — once with a correction, once with an accusation — and to be silent otherwise. Thirty minutes. The observer records every interruption the therapist allowed and every one she prevented, and how she prevented it.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Structure	Questions asked out of order or skipped	All eight asked, some merged	All eight asked in order, with the answers written verbatim
Protecting the individual	Allowed the interruption and the session became a couple argument	Stopped the interruption but lost the client's thread	Stopped it, named why, and returned the client to the exact question

Criterion	0–1	2–3	4–5
Using the partner	Partner ignored	Partner invited at the end	Partner invited at the two designed moments, and her contribution used
The off-time reading	Took the events at face value	Noticed one off-time event	Named the gap between the timetable and the life, in the client's words
The close	Summarised for the client	Checked the summary	Checked it, was corrected, and used the correction

The distinction that carries most of the marks. The top band requires the therapist to write the answers verbatim, which is harder than it sounds: most trainees paraphrase into clinical language within the first question, and the paraphrase is what loses the case. "He says his father would have seen the house" is data. "He reports unresolved paternal expectations" is a summary of the therapist's own theory.

9.9 Six errors

Error 1 — Hearing the relationship and missing the life. The couple's complaint is the presenting system, not the whole person. A therapist who works only with the couple's communication will produce better arguments about the same unmet developmental needs.

Error 2 — Imposing the timetable. Telling a forty-year-old unmarried woman that the social clock is oppressive is not support; it is the substitution of the therapist's analysis for the client's dilemma. Ask about the clock; never set it.

Error 3 — Pathologising the reckoning. A man of forty-five who is grieving a life is not necessarily depressed, and the distinction matters because the treatments differ. The screening question is duration, impairment and severity, as in Week 8 — and if the criteria are met, treat the depression, because a depression will prevent the reckoning being conducted at all.

Error 4 — Romanticising the reckoning. The mirror error. "He is finding himself" is not an adequate response to an affair, an abandoned family, or a resignation that leaves a household without income. The developmental frame explains the behaviour; it does not license it.

Error 5 — Treating generativity as a virtue. Generativity becomes a trap when it is the only structure a person has: the man who is the roof for everyone and receives nothing is not a saint, he is a system with no return path, and the collapse is predictable. The audit's third column is the clinical finding.

Error 6 — Believing the timetable is the same for both partners. Couples routinely run on different clocks — different families, different expectations, sometimes different countries. The two-clock exercise is not an icebreaker; it is the assessment of a mismatch that has been conducted silently for twenty years.

9.10 Culture, faith and the adult timetable

The Nigerian adult clock. The timetable in most of the households this book is written for is explicit and densely populated: finish university by twenty-three, marry by twenty-eight, first child within a year of marriage, a house by forty, a title by fifty, a grandchild by sixty. The clock is enforced by relatives, by church and mosque, and by the ordinary conversation of a compound. An adult who is off it is not merely uncomfortable; they are the subject of a collective project of correction.

The clinical consequences are specific and worth naming. A thirty-four-year-old unmarried woman presents with anxiety that is entirely rational given her circumstances, and the therapist's first task is to distinguish between her own wish and the family's project. A forty-year-old man with no child presents with depression and is asked by everyone he meets when he will have one. A couple with one child and secondary infertility present with a marital problem that is, in fact, a public one.

The first son, the last born, and the birth-order contract. Many households operate an explicit birth-order contract: the first son is responsible for the parents, the first daughter for the younger siblings, the last born for the parents' old age. The contract is not usually written and is rarely negotiated. Dapo's case is the ordinary version, and the clinical move is not to overturn it but to make it speakable — who decided, when, and what is the limit — because an unstated contract cannot be declined and therefore cannot be renegotiated.

Faith and the meaning of a stalled life. Adults in a religious household may interpret a plateau, a childlessness, or a bereavement as a spiritual matter, and the interpretation is frequently a resource rather than an obstacle: it provides a language for endurance and a community for support. The clinical error is to treat the framework as a defence to be interpreted. The workable position is to ask what the framework is doing — sustaining, or postponing — and to negotiate only what must be negotiated, which is usually whether the person is also permitted to be angry.

Migration and the split timetable. A substantial proportion of the adults in these clinics have one foot in another country: a sibling in the United Kingdom who sends money, a spouse working in the Gulf, a child studying abroad. The developmental timetable of the household is then partly external, and the clinician must ask who is where, who is expected to return, and what the migration has cost. A couple's "distance" is sometimes a visa.

> **DEBATE**
> "A forty-five-year-old man who has done everything his family required of him and is now miserable should be encouraged to disappoint them." Argue for: the foreclosure was never his choice, the misery is the cost of a life authored by others, and the therapeutic task is to return authorship. Argue against: the obligations are real, the family is not an obstacle to be overcome, the man is likely to lose more than he gains, and encouraging rebellion is a projection of the therapist's individualism. Then state what you would actually say to Dapo if he told you he wanted to stop paying his brother's school fees.

9.11 Evidence note

What the research supports. The longitudinal evidence for continuity with change is strong: personality traits are rank-order stable across adulthood while their behavioural expression shifts, and the "midlife crisis" as a universal phenomenon is not supported. Generativity in midlife is associated with better wellbeing and, in several cohorts, with better outcomes for the next generation — though the direction of causation is not settled. The association between off-time life events and distress is robust across cultures, which is the strongest single support for the social-clock concept.

What is weaker. Levinson's seasons rest on a small interview sample of American men and should be taught as a heuristic. Vaillant's cohort studies are rich and unrepresentative — educated, male, and longitudinally privileged — and their conclusions travel poorly to households with different structures. The "work as calling" literature is largely correlational and heavily confounded by occupation and income.

What is not supported. That there is a universal midlife crisis with a predictable shape. That marriage has invariant stages that all couples pass through in order. That generativity requires children. And that an off-time life is a damaged one — the evidence is about distress, not about outcome.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same promotion at thirty-eight is a triumph for one manager and a ceiling for another, and what distinguishes them is not the event but what the event was supposed to mean.

9.12 Key terms

Intimacy versus isolation — Erikson's sixth stage; the capacity to be known without managing what is known.

Fusion, pseudo-intimacy, isolation, serial replacement — the four adult forms of avoiding intimacy.

Generativity — concern for establishing and guiding the next generation; the midlife task.

Social clock — the culturally shared timetable for life events; distress follows deviation from it more than the events themselves.

Off-time event — a life event occurring earlier or later than the cultural timetable expects.

Life structure — the underlying pattern of a person's life at a given time: roles, relationships, and the commitments that organise them.

Continuity with change — the finding that personality is stable in structure and changing in expression across adulthood.

Foreclosure — a commitment adopted without exploration, typically inherited from the family.

Birth-order contract — an unstated family allocation of responsibility by ordinal position.

Two clocks — the divergence between partners' expected and actual timetables.

9.13 Self-test

1. Name the four adult forms of avoiding intimacy and one presenting complaint for each.
2. What is the difference between a job, a career and a calling, and why is the distinction clinically useful?
3. Explain the social clock and give three off-time events with their typical clinical presentations.
4. Why does the lecture insist that the reckoning be taken seriously and that the person be held accountable? Give the error that each half prevents.
5. In the Dapo case, what were the four developmental events, and why did they arrive together?
6. What is the third column of the generativity audit, and what does an empty column mean?
7. Why is the partner asked what she gave up, rather than what she wants?
8. What is the difference between pathologising and romanticising a midlife reckoning, and what is the position between them?

Answers. (1) Fusion ("he cannot bear me having a friend"); pseudo-intimacy ("we are very close" with no private conversation); isolation ("he does not let me in"); serial replacement (a pattern of relationships each of which ends when it becomes known). (2) A job is for money, a career is a trajectory, a calling is what you would do anyway; the distinction identifies what a person has actually lost when work changes, which determines whether the intervention is occupational or developmental. (3) A first child at nineteen (interrupted own development); a first marriage at forty-five (enforced singleness and pressure); retirement at fifty-five (shame and somatic presentation). (4) Taking it seriously prevents pathologising a developmental task as a disorder; holding the person accountable prevents romanticising harm as self-discovery. (5) A work plateau, his father's death, his daughter's departure, and his mother's widowhood — they arrived together because midlife concentrates losses of role, as the generation ahead dies and the generation behind leaves in the same decade. (6) What I receive; empty means a system with no return path, and the intervention is to build one. (7) Because the question produces the partner's own developmental account, which converts an assessment of one person into a conversation between two, and because "what do you want" invites a demand while "what did you give up" invites a history. (8) Pathologising treats a reckoning as depression; romanticising treats harm as growth; the position between them is to name the task and hold the person accountable for how they conduct it.

9.14 Reflective task

Write your own lifeline, on paper, with the instruction the case used: the things that mattered, not the things that should have mattered. Then write your own two clocks — when you expected to marry, to have children, to be established, to be settled — and when it actually happened or looks likely to. Then answer one question: which gap have you explained to yourself, and which have you not?

9.15 Further reading

- Erikson, E. The Life Cycle Completed — intimacy and generativity, in the author's own late revision.
- Levinson, D. The Seasons of a Man's Life — read as a heuristic, with the sample in mind.
- Vaillant, G. Triumphs of Experience — the Harvard cohort, and the case for continuity.
- McAdams, D. The Redemptive Self — how adults narrate generativity, and why the story matters.
- Neugarten, B. "Time, age and the life cycle" — the original social-clock paper, still the clearest.

Next: Week 10 — Couple development: the relationship across time.

Week 10 — Couple development: the relationship across time

Couples develop too, and the development is not a metaphor. A relationship has tasks that arrive in a sequence, capacities that are built or not built at each stage, and a trajectory that can be described in advance well enough to be clinically useful.

Phase	Central task	Common failure
Formation	Commitment and identity as a couple	One partner never really chooses
Early marriage	Building rituals and boundaries with families of origin	In-law triangles
Young children	Dividing labour; protecting the couple bond	Co-parenting replaces partnering
Adolescent children	Re-negotiating autonomy, revisiting one's own adolescence	Escalation or abdication
Launching	Re-intimacy after children leave	The "empty nest" that was already empty
Later life	Facing illness, retirement, loss together	Caretaking replaces relating

What the map is for. Not prediction, and not assessment of how far along a couple "should" be. The map is a diagnostic instrument with three uses: it tells the therapist which task is live; it predicts which task is next, which is the basis of preventive work; and it locates the couple's complaint in a sequence, which is often the first time either partner has seen the complaint as a stage rather than a verdict.

What the map is not. It is not a universal sequence. Couples form at twenty and at fifty; some never have children and some begin with them; some skip the launching phase because a parent moves in instead of a child moving out; and a substantial number of the households in these clinics are not a two-person system at all but a network of three or four adults raising children together. The map is a set of tasks, not a set of stations.

Why this week follows adult development. Last week was the person's timetable. This week is the relationship's timetable, and the two are frequently out of step. The most common clinical picture in this week is a couple whose individual clocks have diverged from the couple's clock: one partner is still in the formation task ("have we really chosen each other?") while the other is deep in the young-children task ("who is doing the school run?").

> **PRACTICAL**
> With a peer, draw a couple timeline: five high points, five low points, and one turning point each. Then ask: which developmental transition explains the shape of this line? Most students find that at least two of their five low points sit next to a transition rather than next to a conflict, which is the whole argument of the week.

10.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	Why a couple develops	The difference between a relationship that changes and one that grows	Name the last transition their own family passed through
8–22	The six phases	The tasks, in order, with one failure mode each	Place four couples on the map from a one-line description
22–34	Formation and the unchosen couple	The "one partner never really chooses" pattern and how it presents ten years later	Identify the unchosen partner in two vignettes
34–46	Young children	The division of labour, the disappearance of the couple, and the two-year window	Calculate the couple's weekly non-logistical time
46–58	Adolescence and launching	The re-negotiation, and the couple who outsourced their identity to the children	Ask the "what did you talk about before them?" question in role
58–70	The cascade and the repair trajectory	The behaviours that predict dissolution, and the ones that predict recovery	Code a two-minute argument for the four horsemen
70–82	Rituals	What a ritual is, why couples without them drift, and how to build one	Inventory their own family's rituals
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students arrive with a model of couple therapy as conflict management, and the week's job is to replace it with a model of

development management. The demonstration that does this is to take a couple who argue well — who resolve every disagreement respectfully — and show that they are nonetheless in trouble, because they have not had a non-logistical conversation in four years. Conflict skill is necessary and it is not the same as a relationship.

The sentence the lecture must land. "Couples do not fail because they fight. They fail because the tasks change and nobody tells them, so they keep doing the last stage's work in this stage's conditions."

10.2 The six phases, with the clinical reading of each

1. Formation. The task is commitment: two people becoming a couple that is distinct from the two families that produced them, and each of them genuinely choosing. The clinical issue that matters most is the unchosen couple — the relationship entered because of pregnancy, family pressure, immigration status, loneliness, or the end of another relationship. Ten years later, the unchosen partner presents a resentment that has no argument attached to it, because there was never a decision to point at.

The signature in the room: one partner can describe the moment they chose, and the other cannot. The therapist's question is not "do you love him?" but "when did you decide?" — and the answer, or its absence, organises the whole treatment.

2. Early marriage. The task is building rituals and boundaries: how Sundays work, whose family comes for Christmas, who decides about money, what the couple does that is theirs. The characteristic failure is the in-law triangle — a partner in coalition with a parent against the other partner, usually without intending it.

The signature: the couple's disagreements are not about content but about loyalty, and every argument resolves into a question of whose family wins. The intervention is structural rather than communicative: the couple must have a stated position that they can deliver to both families, in the same words.

3. Young children. The task is dividing labour while protecting the couple bond. The characteristic failure is that co-parenting replaces partnering: the couple becomes an efficient organisation for the management of children and ceases to be a relationship. This is the single most common presentation in couple clinics worldwide, and it is not a failure of love but a failure of time and structure.

The signature: the couple can talk for an hour about the children and cannot name what they did together in the last month that was not for the children. The assessment question is arithmetic: how many hours a week does this couple spend alone together, awake, not discussing logistics? The answer is usually between zero and two.

4. Adolescent children. The task is re-negotiating autonomy as a family, and — unavoidably — revisiting each parent's own adolescence. The characteristic failure is the two patterns from Week 7: escalation and abdication, frequently with one parent in each.

The signature: the couple's conflict is conducted through the adolescent. One parent is the strict one, the other the lenient one, and the adolescent has learned to route every request

through the lenient one. The developmental reading is that the parents are replaying their own adolescent conflicts with each other, using the child as the arena.

5. Launching. The task is re-intimacy after the children leave. The characteristic failure is the empty nest that was already empty: a couple who discover, at the point of launch, that they have not had a relationship independent of the children for twenty years.

The signature: the couple presents when the last child leaves, with a complaint that sounds sudden and is not. The question that opens it — "what did you talk about before them?" — usually produces a silence followed by an accurate and painful answer.

6. Later life. The task is facing illness, retirement and loss together. The characteristic failure is that caretaking replaces relating: one partner becomes a nurse and the other a patient, and the relationship that was between two adults becomes a service.

The signature: the healthy partner describes exhaustion and the ill partner describes loneliness, and both are describing the same loss. The intervention is to restore some domain of the relationship that is not about the illness — one hour, one activity, one subject that is not permitted to be medical.

10.2a What the therapist asks at each phase

The table below is the practical instrument. In a first session, the therapist does not need to know which phase the couple is in so much as which task is live, and the questions below identify it in about ten minutes.

Phase	The question that identifies it	What a positive answer means
Formation	"When did you decide?"	If only one can answer, the couple is unchosen
Early marriage	"Whose family decides about Christmas, and who decided that?"	A coalition with a family of origin is live
Young children	"How many hours a week are you alone together, awake?"	Below two hours, the bond is unprotected
Adolescent children	"Who is the strict one?"	The conflict is being routed through the child
Launching	"What did you talk about before the children?"	The couple's identity was outsourced
Later life	"When did you last do something together that was not about the illness?"	Caretaking has replaced relating

The rule for using the table. One question per session, not six. A therapist who runs the whole table in the first session produces a couple who feel assessed rather than met, and the information gained is worth less than the alliance lost. The table is for the therapist's internal map; the questions are delivered one at a time, across sessions, in the order the couple's material invites.

10.2b The cascade and the repair trajectory

Couples do not dissolve in a single event. They move along one of two trajectories, and the behaviours that distinguish them are observable in a two-minute argument.

The cascade. Four behaviours reliably predict deterioration, and they are not the loud ones. They are: criticism (an attack on the person rather than the behaviour: "you always", "you never"); contempt (mockery, sarcasm, eye-rolling, name-calling — the single strongest predictor); defensiveness (counter-attack or victim stance in response to a complaint); and stonewalling (withdrawal, silence, a phone). These are usefully taught not as sins but as stages of a losing strategy: criticism is a complaint that has been made too many times without being heard; contempt is criticism that has stopped expecting an answer; defensiveness is the response of a person who believes the accusation is a verdict; and stonewalling is what a physiologically flooded person does when they have no other option.

The repair trajectory. The distinguishing feature of couples who recover is not the absence of the four behaviours but the presence of repair attempts: a joke, a hand, a "can we start again", a break, a sentence that concedes one point. Repair attempts succeed in proportion to the couple's capacity to notice them — which is trainable — and in proportion to the positive sentiment override built up outside the argument.

The developmental reading. The cascade is what happens when a stage's task is not done and the couple keeps using the previous stage's methods. The couple in the young-children phase who are still arguing like a formation couple — about whether they chose each other — will fight about the school run and mean something else entirely. The therapist's job is not to teach them to argue better about the school run. It is to identify which task is live and to move the argument there.

What predicts recovery, in order. The couple's capacity to repair in the moment; the ratio of positive to negative interaction outside conflict; the presence of shared meaning (children, faith, a project, a place); and the absence of contempt. Note what is not on the list: the frequency of conflict, the content of the disagreement, and the couple's skill at "communicating". Many couples with excellent communication skills are in trouble, and many couples who shout are not.

10.3 The method: the couple timeline

The timeline is the assessment and the intervention at once, and it takes one session.

Step 1 — Draw it, with both present, on one sheet. A horizontal line, and the years along it from the couple's meeting to today. Above the line: five high points. Below: five low points. Both partners contribute; the therapist writes.

Step 2 — Ask for the same five moments from each partner separately, then compare. The comparison is the finding. Couples routinely produce different high points, and the difference is not a memory error; it is a difference in what each of them is living for.

Step 3 — Mark the transitions in a third colour. Marriage, first child, a move, a job change, a bereavement, a migration, a child leaving. Then look at the line. The pattern that appears in the great majority of cases is that the low points cluster adjacent to the transitions, not adjacent to the arguments.

Step 4 — Ask the turn question of each. "Which point on this line was the turning point, and what turned?" The answers are frequently surprising and always usable, and they are the material for the rest of the treatment.

Step 5 — Ask the unanswered question. "What happened on this line that is not on it?" The question produces the events the couple has agreed not to discuss: an affair, a miscarriage, a debt, a termination, a period of violence, a child who died. The therapist asks it once, and does not pursue the answer in the first session.

Step 6 — Convert the timeline into one task. Not a programme. One thing that the couple will do, or stop doing, between now and the next session, chosen because it addresses the live task rather than the loudest complaint.

Why the technique works. It externalises the relationship. A couple who have been conducting a characterological argument ("you are selfish") are suddenly looking at the same object on a page, and the object is a history rather than a defect. It also does something the partners cannot do alone: it makes visible the ratio between the good years and the bad, which is frequently more balanced than either partner believed.

10.3a The turning-point interview

The turn question from step 4 deserves a section of its own, because it is the most productive single question in couple work.

The question. "If you had to put your finger on one point where this relationship turned — for better or worse — which point would it be?"

Why it works. It asks for a moment rather than an evaluation. Couples in difficulty have an evaluation ready ("it has been bad for years") and no moment, and the request for a moment produces specificity, a date, and usually an incident that the other partner did not know was being carried.

The four kinds of answer, and what each requires.

- **An event** — a job loss, a move, a death, an affair. The therapist's work is to establish what the event meant, which is usually not what happened but what was not said about it.
- **A decision** — "when we decided to have the second child", "when I took the posting". The work is with the decision's authorship: who decided, and who was informed.
- **A sentence** — "when he said, in front of his mother, that I was exaggerating". The work is with the moment itself, which is frequently the first time the other partner has understood its weight.
- **A silence** — "I cannot think of one". This is the most informative answer of all, and it usually means that the couple's difficulty is chronic rather than eventful, which changes the treatment from a reconstruction of an injury to a construction of a relationship.

The rule for the therapist. Do not choose between the two answers. A couple will offer two turning points and the therapist's temptation is to adjudicate. The useful move is to

ask each partner what the other's turning point means to them, which is the first genuinely mutual act in most of these sessions.

10.4 Three techniques

Technique 1 — The ritual inventory. List every recurring practice in the household: the Sunday meal, the Friday call to a parent, the way birthdays are marked, the prayer before a journey, the walk, the programme watched together, the way an argument ends. Then mark each as alive, dead, or contested. Couples in difficulty have reliably lost their rituals, not their love, and the restoration of one ritual is frequently more effective than a communication exercise.

Technique 2 — The division-of-labour audit. Two columns: everything that has to be done in a week, and who does it. Then a third column, which is the one that matters: who notices when it is not done. The third column is the measure of mental load, and it is the source of most of the resentment in the young-children phase. The audit is not a negotiation; it is a description, and the description usually produces the negotiation without the therapist having to run it.

Technique 3 — The twenty minutes. One twenty-minute period per week, at a stated time, with no children, no money, no logistics and no problem-solving. The rule that makes it work: if a problem is raised, the twenty minutes ends and the problem goes to a different slot. Couples find this harder than any other task in the book, and their difficulty is the diagnosis.

10.5 The first session with a couple in the young-children phase

Phase 1 — The complaint, in each person's words, with one example each (15 minutes). Written down verbatim. The therapist does not allow the partners to respond to each other's examples in this phase, and says why: "you will each get to answer, and I want the examples to be on the table first."

Phase 2 — The timeline (25 minutes). Drawn on one sheet, both contributing, with the transitions marked in a third colour.

Phase 3 — The live task, named and checked (5 minutes). "I think the task in front of you is not the argument about the phone. I think it is that you have not been alone together, awake and not talking about the children, for about two years. Is that right?" The couple corrects, and the corrected version is the formulation.

Phase 4 — The arithmetic (5 minutes). "How many hours a week?" The answer is written on the sheet, next to the timeline, because a number on a page is more persuasive than a number in a conversation.

Phase 5 — One task (10 minutes). The twenty minutes, scheduled now, in the session, in the diary. The therapist does not leave the scheduling to the couple, because the couples who most need it are the ones least likely to find the time.

Why the timeline precedes the task. A couple given a task before they have seen their own history experiences the task as an instruction from a stranger. A couple who have just

drawn twenty years on a page experience the same task as a repair to something they can see.

10.5a When one partner will not come

The commonest structural obstacle in couple work: one partner attends and the other refuses. Three positions, and one of them is right.

The wrong position — treat the attending partner individually as if the couple were the problem. This produces a therapy that is, in effect, an alliance against an absent person, and the absent person will eventually be told what was said, accurately or otherwise.

The second wrong position — refuse to work until both attend. This abandons the person in front of you and, in practice, ends the treatment.

The workable position — work with one, on the relationship, explicitly and transparently. Say what the frame is: "I will work with you on what you can change, which is your part. I will not build a case against your husband." Then three practical moves: ask the attending partner to invite the other once, in the therapist's words, with the offer that the first session is a consultation and not a commitment; offer a single joint session with an explicit agenda that the absent partner can read in advance; and set a review date at which the situation is reassessed.

The question that changes the refusal. An absent partner usually refuses for one of three reasons: they believe the problem is the other person's; they are ashamed; or they are afraid of what will be said. The invitation that works addresses the third: "nothing will be decided about you in a room you are not in." That sentence, delivered by the attending partner, converts a substantial minority of refusals.

10.4a The negative cycle, in the couple's own words

Emotionally focused couple therapy describes a circular pattern — a pursue-withdraw or attack-defend cycle — that organises most distressed couples' arguments. The description is useful to a developmental therapist because the cycle is the mechanism by which a missed stage task turns into a stable arrangement.

How to name the cycle without jargon. Ask each partner two questions: "when you are not getting through, what do you do?" and "what do you see him doing?" The answers produce the cycle in the couple's own language, which is the only version that will be remembered. A typical formulation, in a couple's words: "when I feel alone I ask for more, and the more I ask the more he goes quiet, and the quieter he goes the more I ask." The therapist writes it down and gives it a name the couple chooses.

Why the developmental frame matters here. The cycle is not a communication defect; it is a stage-specific adaptation that has outlived its stage. In the young-children phase, pursuing is what one partner does to protect the bond from disappearing and withdrawing is what the other does to survive exhaustion. Both strategies are reasonable responses to the actual conditions. They become a cycle when each is experienced by the other as the

cause of the problem, and the cycle becomes stable when neither partner can see their own half of it.

The three moves that break it, in order.

First, name the cycle as the problem, not the partner. "The thing that has been happening in your house for four years is this pattern, and it is not either of you." This is not a rhetorical softening; it is accurate, and it changes the couple's task from mutual accusation to joint observation.

Second, find the primary emotion under each position. The pursuer's anger is usually underlain by fear of being unimportant; the withdrawer's flatness by a conviction of failure. Naming these is done with tentativeness — "I may be wrong, but I wonder whether the quiet is not indifference but a kind of protection" — because a confident attribution at this point is experienced as taking the other's side.

Third, give one small enactment in the session. Ask the pursuer to say the fear directly to the withdrawer, and the withdrawer to answer without defending. The therapist keeps it to two turns and stops it before it becomes a conversation, because the first two turns are the intervention and the third is usually a relapse.

What the developmental therapist adds. The cycle is described, and then the task is addressed: the Thursday, the audit, the defined grandmother. Naming a cycle without changing the structure that produces it produces a couple who understand their pattern and continue to enact it, which is the characteristic failure of insight-oriented couple work in a household with no time.

10.5b The couple with a child with a disability

One presentation deserves separate treatment because it reorganises the couple's developmental timetable more than any other.

What changes. The young-children phase does not end when it should; the launching phase may never arrive; the division of labour becomes permanent and unequal; the couple's social world narrows to families in the same situation or disappears; and the services system becomes a third party with its own demands, appointments and forms. The couple's shared meaning is tested in a way that few other events test it.

What the evidence says. The divorce rate among parents of children with disabilities is frequently quoted as higher than the general population; the better-designed studies find a smaller and less consistent difference than the popular claim, with the strongest predictors being the child's behaviour difficulties and the family's financial and service support rather than the disability itself. What is robust is the elevation of parental stress and the reduction of couple time.

The three clinical tasks. First, protect one domain of the couple's life that is not about the child — which is the same intervention as the twenty minutes, with a higher stake. Second, define the division of labour explicitly, because the default is that one parent becomes the specialist and the other becomes the provider, and the arrangement is rarely

chosen. Third, address the sibling, who is the family member most often described as "fine" and least often asked.

The question that opens the work. "What did you imagine this would be like, and what is it like?" Asked of each partner separately, then together. The gap between the two answers is the couple's grief, which is usually unmourned because the child is loved and the grief feels like a betrayal of the child. Naming that directly — "it is possible to love this child completely and to grieve the life you expected" — is frequently the most useful sentence in the treatment.

10.6 Case: Aisha and Sam, four years on

The couple first met in Week 2, where Aisha, 31, and Sam, 34, were assessed for goodness of fit with a temperamentally slow-to-warm infant. They return three years later with a second child.

The household. Aisha, 34, a lawyer, on a partnership track. Sam, 37, an architect, in a small practice he started with a colleague. Their daughter, Yusra, is four and in pre-school; their son, Bilal, is nine months. Sam's mother, 66, comes three days a week to help. Aisha's sister lives twenty minutes away and has two children of her own.

The referral. Aisha telephones. "I do not think I love him any more. I am not saying I want to leave. I am saying I cannot remember." Sam's version, in the first session: "she is never here. When she is here she is on her phone. She has decided something and she has not told me what it is."

The complaint, with examples. Aisha's example: "last Saturday I came back from the market and he had taken Bilal to his mother's without telling me, and I sat in the car outside my own house for ten minutes because I did not want to go in." Sam's example: "on Wednesday she came home at ten and I had done both bath times and she said 'you should have put him down earlier'."

The timeline. Drawn with both, and it is the session.

- 2016 — met at a wedding. High.
- 2018 — married. Aisha's family objected to the tribe; she married him anyway. High.
- 2019 — Yusra born, difficult birth. Low.
- 2020 — Sam's practice started; the money stopped for eighteen months. Low.
- 2021 — Aisha's father died. Low, the deepest mark on the line.
- 2022 — Aisha made senior associate. High.
- 2023 — The move to the bigger flat. High, contested.
- 2024 — Bilal born. High and low in the same month.
- 2025 — Sam's practice stable; Aisha's partnership decision due. Unmarked, so far.

What the timeline shows that neither partner had said. Three findings.

First, the low points cluster at 2019 to 2021, and the couple has never discussed 2021 at all. Aisha's father died in the same year Sam's practice was failing and Yusra was

two. The therapist asks the unanswered question — "what is not on this line?" — and Aisha says: "I was pregnant in 2021 and I lost it in the August, and then my father died in the October, and nobody in this family knows that because we were doing the practice." Sam puts his face in his hands. It is the first time in four years that this has been said in the same room as both of them.

Second, the high points are almost all about achievement, and none of them are about each other. 2022 is Aisha's promotion. 2023 is the flat. The couple's celebrations are external, and the therapist names this without interpretation: "there is nothing on the top of this line that is just the two of you."

Third, the word "partnership" appears twice — Sam's practice and Aisha's track — and the couple is competing in the same currency. They are not in conflict about love. They are in a symmetric struggle in which each is asking the other to accommodate a career, and neither has said so in those terms.

The live task. The young-children task: dividing labour while protecting the couple bond. The arithmetic is performed in the session: Aisha estimates that she is alone with Sam, awake, not discussing children, for "about twenty minutes a week, and that includes the argument". Sam says "less".

The three findings that produce the formulation.

1. **The couple's bond was never built as a separate structure; it was built as a joint project.** Two ambitious people married in 2018 and immediately began building: a practice, a career, a flat, two children. The project is now succeeding and the relationship that was supposed to be its purpose has no independent existence. This is not a failure of love; it is a failure of construction.
2. **An unmourned loss has been silently organising the last four years.** The miscarriage and the father's death in the same autumn were absorbed as a logistical problem ("we were doing the practice") and never mourned. Aisha's "I cannot remember loving him" is accurate: the years in which the memory would have been made were the years in which the household was in emergency.
3. **The conflict about the market and the bath time is not the conflict.** It is the ordinary surface of a symmetric struggle about whose career accommodates whose, conducted through domestic detail because neither partner has the standing to raise the real question — which is that they have never decided whose work comes first.

The intervention, in five parts.

Part 1 — Name the mourning, once, and do not process it in this session. The therapist marks the September and October on the timeline, says "that is not a bad year, that is two deaths and a birth plan that ended", and moves on. The instruction is to notice, not to work. A couple who have avoided something for four years will not process it in the session in which it is first named, and a therapist who tries will teach them to avoid it more carefully.

Part 2 — Build one structure that is not the project. The twenty minutes, scheduled in the session, on Thursday at nine-fifteen, after Bilal's last feed. The rule: no children, no

money, no problems. The couple's objection — "we have nothing to say" — is answered by predicting it: "the first two will be awkward. That is the finding, not the failure."

Part 3 — The division of labour audit, with the third column. Written in the session. The third column — who notices when it is not done — is almost entirely Aisha's, and Sam's reaction to seeing it written is the session's second significant moment. The negotiation that follows is not run by the therapist; it is run by the list.

Part 4 — The career question, named as a question. "Whose work comes first when they conflict?" is asked plainly and answered badly, which is the beginning. The couple agrees on a rule for one year: Aisha's partnership year takes precedence for childcare logistics, and in exchange Sam's practice gets the first claim on any surplus money and one weekend a month. The rule is arbitrary and it is written down, and the arbitrariness is the point: a rule they chose is different from a pattern they resent.

Part 5 — The grandmother, defined. Sam's mother comes three days a week to help. The therapist asks what she decides and what she does not. The answer reveals that she has views about Bilal's feeding which she expresses to Sam and not to Aisha, and that Sam has been carrying them. The three-domain agreement from Weeks 3 and 7 applies unchanged: what she decides, what she does not, and the sentence that says so in front of everybody.

10.6a Four months later

The Thursday twenty minutes has happened eleven times out of sixteen. Aisha reports that the first three were "terrible" and that the fourth was the first conversation in four years in which nobody mentioned a child.

The miscarriage has been spoken about twice more, both times on a Thursday, both times briefly. The therapist has not run a grief protocol and does not intend to: the couple is doing the work in the place where the relationship is being rebuilt, which is more durable than a protocol.

The rule about careers has already been broken once, in Aisha's favour, and renegotiated without a session.

Sam's mother now says her views about feeding to both of them, which is worse in the short term and better in the long term. Aisha reports this with irritation and the therapist logs it as progress.

The honest limitation. Aisha has not said she loves him. In the final session she says, "I do not want to leave", which is a different sentence and is the one available at this stage. The therapist does not convert it into a declaration, and the case is closed with a review at six months.

What would have changed the plan. If the answer to "what is not on this line?" had been an affair, the sequence would have been disclosure, then a decision about the frame, then the developmental work. If either partner had disclosed violence — which the therapist screened for in the first session, as a matter of routine — the couple work would have stopped and safety work begun. If Aisha had said "I have decided", the treatment

would have become a separation-focused process, which is a different treatment with a different endpoint and a different literature.

10.7 Transcript: the unanswered question

Ten turns from the third session, at the point where the timeline has been drawn and the therapist asks what is not on it. The couple has been together nine years.

> **T1 Therapist:** I want to ask one more question about the line, and then we will stop drawing. (pause) What happened in this relationship that is not on it? — The question is asked once, flatly, after the couple has invested twenty minutes in the drawing, which is when they have something to protect and therefore something to add. **T2 Aisha:** (long silence) Nothing. It is all there. **T3 Therapist:** (waits) — The single most important behaviour in the transcript. The silence is not a technique for producing disclosure; it is the absence of pressure, which is the only condition in which disclosure occurs. **T4 Aisha:** (quietly) There is one thing. (pause) I was pregnant in 2021. In the August. And then I was not. **T5 Sam:** (turns to look at her) What? **T6 Therapist:** (to Sam, very quietly) You did not know. — Not a question. A statement that gives the partner a moment to be a person rather than a defendant. **T7 Sam:** (shakes his head) **T8 Aisha:** You were at the practice. The money had gone. My father was already ill. I thought — (stops) I thought if I told you it would finish you. — The reason for the silence, which is always a protection and never a deception, and the therapist's next move depends on recognising that. **T9 Therapist:** (to both, after a pause) I am not going to work on this today. I want it on the line, and I want you to know that it is on the line. We will come back to it when you have had a week to know that it is known. — The refusal to process. Almost every trainee wants to work this material immediately, and immediate processing of a four-year silence teaches the couple that disclosure produces a session rather than a relationship. **T10 Sam:** (to Aisha, not to the therapist) I would have wanted to know.

10.8 Laboratory: the couple timeline

Aim. To draw a couple timeline in twenty-five minutes, identify the live developmental task, and convert it into one task without running a communication exercise.

Procedure. Trios: therapist, two partners. The trainer briefs the couple with a nine-year history containing three transitions and one undisclosed event which one partner will reveal only if asked the unanswered question at the right moment. Twenty-five minutes, then ten minutes of feedback.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The drawing	Therapist drew it, or asked for a verbal history	Couple contributed, but unequally	Both partners' marks on the page, in the therapist's handwriting, with transitions in a third colour
The comparison	Treated the two lists as one	Noticed a difference	Used the difference as the finding and asked what each difference meant

Criterion	0–1	2–3	4–5
The transitions	Not marked	Marked, not used	Marked, and the clustering of lows named
The unanswered question	Not asked	Asked, then pursued immediately	Asked once, held, and the answer scheduled rather than processed
The task	A programme	One task, unscheduled	One task, scheduled in the session, with a prediction of how it will fail

The distinction that carries most of the marks. The top band requires the therapist to predict the failure of the task she has just set. A couple told that the first three Thursdays will be awkward is protected from concluding, on the first awkward Thursday, that the therapy is not working.

10.9 Six errors

Error 1 — Teaching communication skills to a couple whose problem is structural. A couple with no shared time, two careers and an undefined grandmother do not need to learn to listen; they need a Thursday. Skill without structure produces a couple who listen beautifully in a session they cannot schedule.

Error 2 — Treating the loudest complaint as the task. The market and the bath time are the surface. The therapist who negotiates the surface will be negotiating it again in a month, with a different example.

Error 3 — Taking sides with the more articulate partner. In every couple there is one who speaks in paragraphs and one who speaks in sentences, and the therapist's sympathy follows the paragraphs. The correction is arithmetic and it should be visible: equal turns, equal minutes, and the therapist counting.

Error 4 — Processing a disclosure immediately. The transcript above is the exemplar. A four-year silence is not resolved in the session in which it ends; it is registered, held, and returned to.

Error 5 — Believing the "empty nest". A couple who present at the point of the last child's departure have usually been in difficulty for a decade. The presentation is late, not sudden.

Error 6 — Ignoring the household beyond the couple. Three adults raising two children is not a couple with an in-law problem; it is a caregiving network with an undefined hierarchy. The unit of treatment is whatever is actually raising the children, which is frequently more than two people.

10.10 Culture, faith and the couple

Marriage as a contract between families. In much of the world the couple's formation is not a private decision but a treaty: bride price, family approval, religious ceremony, and obligations that run in both directions for decades. The Western developmental map's "formation" phase, with its emphasis on the couple choosing each other, describes one version of a universal task. In a marriage arranged with family involvement, the task is not choosing but authoring — the couple must become the authors of a relationship that others initiated, and the clinical question is whether they have ever been given the opportunity.

Polygyny and the couple unit. Where a household includes more than one wife, the couple is not the clinical unit and a therapy designed for two people will misdescribe the system. The relevant tasks are allocation (of time, money, and the husband's presence), hierarchy (whose children, whose room, whose authority), and the children's experience of the arrangement. The therapist who has a position on polygyny will be unable to work; the therapist who can map the allocation without endorsing or condemning it can.

Religion as a third party in the room. Couples frequently hold their marriage inside a religious framework that supplies obligations, rituals and an audience. This is a resource: it supplies a shared meaning, which is on the list of what predicts recovery. It is also a constraint: a couple who believe divorce is unavailable may present late, when the difficulty is chronic. Both are facts to be assessed, not positions to be taken.

The wife who earns more. A specific and increasingly common presentation in the households this book is written for: a wife whose income exceeds her husband's, in a culture where provision is part of the definition of masculinity. The clinical picture is not usually the couple's own discomfort; it is the discomfort of the wider family, conducted through remarks, and it produces a husband who withdraws from decisions and a wife who resents the withdrawal. The intervention is to name the external audience and to build the couple's own position on it, which is Week 10's early-marriage task arriving late.

> **DEBATE**
> "A couple should stay together for the children." Argue for: the evidence on children's outcomes is about conflict, not separation, and a low-conflict marriage is a better environment than a divided household in most circumstances. Argue against: a high-conflict marriage is worse for children than a clean separation, the phrase is used to trap people, and the children in question learn the relationship they are shown. Then state what you would actually say to a couple who tell you this is their reason for staying, and what you would ask them first.

10.11 Evidence note

What the research supports. The observational literature on couple interaction is strong: contempt, criticism, defensiveness and stonewalling predict deterioration, and repair attempts predict recovery, with effect sizes that survive controls for initial satisfaction. The association between shared meaning and relationship stability is consistent across studies. Emotionally focused couple therapy has the best evidence base of the couple modalities, with replicated effects in randomised trials and follow-up to two years (MFT 710).

What is weaker. The stage models. The six-phase map is a teaching device assembled from family life cycle theory and clinical observation; it is not a validated developmental sequence, and couples vary enormously in the order and timing of the tasks. The claim that the couple should be the primary unit of treatment is a value as much as a finding, and in households with distributed caregiving it is frequently wrong.

What is not supported. That communication training alone improves satisfaction in distressed couples — the evidence is that skill acquisition without change in the couple's structure and emotional bond does not transfer. That the empty nest is a crisis for most couples — the average finding is an improvement in satisfaction after the last child leaves, for couples who were not already distressed.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same second child, born into a couple with a Thursday and a defined grandmother, is a manageable strain; born into a couple with no shared time and an undefined authority, it is the point at which the relationship loses its structure entirely.

10.12 Key terms

Formation — the phase in which the couple becomes a distinct unit and each partner chooses it.

The unchosen couple — a relationship entered without a genuine decision; presents later as resentment without an argument.

Ritual — a recurring, meaningful practice that marks the relationship's continuity.

Negative sentiment override — the state in which a partner's neutral behaviour is read as hostile.

Repair attempt — any move that interrupts an escalating exchange; the best single predictor of recovery.

The four horsemen — criticism, contempt, defensiveness, stonewalling.

Mental load — the noticing and planning of household work, distinct from its execution.

Symmetric struggle — a conflict in which both partners want the same thing from each other, and neither can yield.

Shared meaning — the couple's common account of what their life is for; a predictor of stability.

Couple timeline — the drawn history of high points, low points and transitions.

10.13 Self-test

1. Name the six phases and one failure mode for each.
2. Why is the "unchosen couple" the most consequential formation pattern, and what is the diagnostic question?
3. What is the arithmetic question of the young-children phase, and what does a number below two hours predict?
4. Distinguish the cascade from the repair trajectory, and name the single strongest predictor in each direction.
5. Why does the therapist refuse to process the miscarriage in the session in which it is disclosed?
6. What is the third column of the division-of-labour audit, and why does it matter more than the first two?
7. How would you work with a couple in which one partner will not attend?
8. In the Aisha and Sam case, what were the three findings, and which one changed the treatment?

Answers. (1) Formation — one partner never chose; early marriage — in-law triangle; young children — co-parenting replaces partnering; adolescent children — escalation or abdication; launching — the nest that was already empty; later life — caretaking replaces relating. (2) Because there was never a decision to point at, so the resentment has no argument attached and cannot be negotiated; the question is "when did you decide?" (3) Hours per week alone, awake, not discussing logistics; below two predicts a bond with no maintenance and a slow loss of the couple's separate existence. (4) The cascade is criticism, contempt, defensiveness and stonewalling; the repair trajectory is the presence of repair attempts. Contempt is the strongest predictor of dissolution, repair the strongest of recovery. (5) Because a four-year silence cannot be resolved in the session in which it ends, and attempting it teaches the couple that disclosure produces a session rather than a relationship. (6) Who notices when it is not done — the measure of mental load, which is the actual source of the resentment. (7) Work with the attending partner on their part, be explicit that no case will be built against the absent partner, invite once in the therapist's words with a consultation-only offer, and set a review date. (8) The bond was built as a joint project rather than a separate structure; an unmourned loss had organised four years; and the conflict was a symmetric struggle about whose career accommodates whose. The first changed the treatment, because it moved the work from conflict to construction.

10.14 Reflective task

Draw the timeline of a couple you know well — your parents, relatives, or your own if you have one — with five high points and five low points, and mark the transitions. Then answer: which transition produced the largest low point, and what did the couple do about it? Most students find that the couple did something practical and nothing relational, which is not a failure; it is what most couples do, and it is the reason this week's techniques are so simple, and so easy to forget in a room where two people are angry

10.15 Further reading

- Gottman, J. What Predicts Divorce? — the observational literature behind the cascade.
- Johnson, S. Hold Me Tight — the emotionally focused account of the negative cycle, written for couples.
- McGoldrick, M. et al. Genograms — the family life cycle chapters that this week's map draws on.
- Carter, B. and McGoldrick, M. The Expanded Family Life Cycle — the source of the phase structure.
- Fowers, B. "The limits of communication training" — why skills without structure do not transfer.

Next: Week 11 — Parenting across stages: talking to parents well.

Week 11 — Parenting across stages: talking to parents well

Parents do not need lectures; they need **accurate, specific, hopeful information in plain language**, and they need it from someone who is not judging them. That is a skill, it is teachable, and it is assessed in this course.

The four parts of the skill.

1. **Normalise first.** "What you are describing is very common at this age."
2. **Give one developmental fact.** "At four, the part of the brain that manages impulse is still being built, so he cannot yet do what you are asking consistently."
3. **Give one concrete action.** "Try naming the feeling and offering two acceptable choices."
4. **Set a review.** "If this is still happening in three weeks, or if X or Y appears, come back."

What to avoid. Technical vocabulary; predictions ("he will grow out of it"); blame; over-reassurance when the concern is real; and overwhelming a parent with five recommendations.

Why this week is the hinge of the course. Every previous week has produced information a parent needs. This week is about delivering it. A therapist with an accurate formulation and a delivery that shames the parent will produce a family that does not

return, and the formulation will have been worth nothing. The skill is not a soft addition to clinical work; it is the mechanism by which clinical work reaches the people who do it.

The developmental spine of the week. The same four parts are used at every age, and the content changes with the stage. Every previous week supplies the content: Week 3 for the infant (synchrony, the still face, the meaning of a cry), Week 4 for the toddler (the limit formula, the language differential), Week 5 for the preschooler (play, pretend, the false-belief shift), Week 6 for the school-age child (industry, monitoring, the report card), Weeks 7 and 8 for the adolescent (privacy, negotiation, risk). This week is where the course becomes usable in a room with a worried parent.

> **PRACTICAL (OBSERVED)**
> A ten-minute parent-feedback role-play using the four parts, with a peer playing a parent who is frightened and slightly defensive. The exercise is assessed (see 11.5), and the standard is not warmth; it is **accuracy plus one action plus a review**.

11.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	Why advice fails	The difference between information and instruction; why parents have already had advice	List the advice they have given and been ignored
8–20	The four parts	The structure, demonstrated on a toddler case	Identify the four parts in a recorded feedback
20–34	Normalising without dismissing	The two errors: "it is normal" as a brush-off, and alarm as a default	Rewrite three dismissing sentences
34–46	The developmental fact	Which fact to give at which age, and how to say it without jargon	Choose the fact for four ages and say it in one sentence
46–58	One action	Why one, and how to choose it	Choose one action for four cases and defend it against a rival
58–70	The review	Setting a review that is not a threat and not a promise	Write three review sentences
70–82	When the parent disagrees	The four reasons a parent rejects a formulation, and the move for each	Role-play a rejection and hold the frame
82–90	The assessment	How the observed role-play is marked	Draft their role-play plan

The teaching decision that makes the week work. Trainees default to either reassurance or instruction, and both are ways of ending a conversation. The lecture's job is to make the conversation open — to leave the parent with something to do and a reason to come back — and the way to do that is to demonstrate the failure first: the lecturer plays a therapist who reassures a worried mother for four minutes, and the class watches the mother's face close.

The sentence the lecture must land. "A parent who leaves your room feeling stupid will not do what you suggested, and will not come back to tell you it did not work."

11.2 Five concepts that organise the week

1. Parenting is a developmental task that changes. The parent has a task at each stage, and the tasks conflict. The parent of a toddler must hold limits; the parent of a seven-year-old must let go of the minute-by-minute view; the parent of a fifteen-year-old must negotiate. Parents who are given information appropriate to their child's age rather than to their own experience are being given something they cannot get from their own upbringing, which is why the developmental fact is the second part of the structure.

2. Information is not advice. Advice tells a parent what to do; information tells them how something works. Advice invites compliance or resistance; information invites thinking. A parent told "you should not shout" has been advised and will feel judged. A parent told "at four, a child's braking system is still being built, which is why shouting produces a pause rather than a change" has been informed, and can then decide what to do. The clinical discipline is to give information and let the parent derive the action — and to supply the action only if they cannot.

3. Normalising is a technical act, not a kindness. "That is very common" does three things: it lowers the parent's shame, which is what prevents disclosure; it locates the child in a population, which changes the parent's attribution from character to development; and it creates permission for the parent to say the next thing. It fails when it is used to close a conversation ("all children do that") or when it is untrue.

4. One action, chosen for feasibility. Five recommendations produce none. The action must be small enough to survive a bad week, specific enough to be observable, and chosen with the parent rather than for them. The test question is: "could you do this on the worst evening of the month?" If not, it is not the action; it is the aspiration.

5. The review is part of the treatment. A review date converts a suggestion into a plan, gives the parent permission to fail, and creates the conditions for the therapist to learn whether anything worked. The review should name the exception ("if he is still doing this in three weeks, or if he starts X, come back"), because an unconditional review is heard as "come back when you have failed again".

11.2a The four parts, worked at five ages

The table gives the same structure five times, with the content drawn from the weeks that produced it. It is the single most useful page in the module and it should be learned well enough to be used without notes.

Age	Normalise	The developmental fact	One action	The review
6 months	"Waking at night is very common at six months, and it is not a sign you have done something wrong."	"At this age he is learning that you still exist when he cannot see you, and that is why he calls."	"Go to him, keep the room dark and boring, and put him down drowsy rather than asleep."	"If it has not changed in three weeks, or if he stops feeding well, come back."
2 years	"Every child this age says no; it is the age, not your parenting."	"He has just discovered that he is a separate person, and 'no' is how he tests it."	"Name the feeling, hold the limit, offer two acceptable choices."	"Three weeks. If the hitting is worse, or if he stops talking, come back sooner."

Age	Normalise	The developmental fact	One action	The review
4 years	"The lying is extremely common at four and it is not a moral problem yet."	"At four he cannot always tell the difference between what happened and what he wishes had happened."	"Ask 'what happened next?' rather than 'why did you do it?'"	"Three weeks, and if the lying comes with other changes — sleep, appetite, friends — come back."
7 years	"Wanting to be first is normal at seven; the comparison has just arrived."	"At seven he begins to compare himself with other children, and losing becomes a fact about him rather than about the game."	"Name the effort rather than the result, once a day, in a sentence he can hear."	"A month. If he stops wanting to play at all, come back."
15 years	"Not talking to you is common and it is not permanent."	"At fifteen the private world is a developmental need, not a rejection; secrecy is what privacy becomes when there is no agreement."	"One question a day, at a stated time, and not at the door."	"Two weeks. If he stops seeing his friends altogether, come back at once."

How to use the table without sounding like a table. Do not deliver the four parts in order as a script. Deliver the normalising sentence, then wait, then give the fact in the parent's own vocabulary, then the action. The structure is for the therapist's head; the parent should hear a conversation.

11.2b The five questions parents are actually asking

Underneath almost every parental question is one of five concerns, and a therapist who answers the question asked without hearing the concern underneath will be asked it again in different words.

1. "Is something wrong with my child?" The question is about diagnosis and it is asked in almost every first session, in some form. The answer that works is not "no" and not "we will see", but a description of what is and is not yet known: "what you are describing is common at this age; the things that would make me worried are X, Y and Z, and he does not have them. I would be looking for them at the review."

2. "Is it my fault?" The question is about blame, and it is usually unspoken. A therapist who does not address it will find the parent either defensive or collapsed, and neither can implement an action. The sentence that works is factual rather than absolving: "children's development is affected by many things, and a parent who is asking this question is usually doing more than they think." Then move to the action, because blame is not resolved by argument but by competence.

3. "Will it get better?" The question is about the future and it is a request for a prediction. Do not predict. Offer the shape instead: "the pattern you are describing usually changes as language develops, which is in the next year. What I cannot tell you is how much, and I would rather review it in three weeks than guess."

4. "What do I do tonight?" The question is about the immediate. It is the most answerable and the most frequently unanswered, because clinicians are trained to formulate and parents need an action before bedtime. Answer it, even if the answer is provisional.

5. "Do you think I am a bad parent?" The question is about judgement, and it is asked by the parents who are most competent and most exhausted. The answer must be honest and behavioural: "I think you are the person who noticed, brought him here, and is prepared to try something different, which is what the parents I worry about do not do."

The rule. Answer the question underneath, and answer it in the first ten minutes. A parent whose underlying concern has been met can then hear information; a parent whose concern is still unanswered will hear every subsequent sentence through it.

11.3 The method: a parent feedback conversation, ten steps

Twenty minutes, in a room with one or two parents, after an assessment. The structure is fixed because improvisation is what produces the two failure modes: a lecture, or a reassurance.

Step 1 — Ask what they think is happening. Before saying anything. The parent's own formulation is the starting point, and it identifies which of the five questions is live.

Step 2 — Ask what they have already tried. Their attempts are the first resource of the plan and the first evidence of their competence. Name one aloud: "you have already done the hardest part, which is changing what you do at bedtime."

Step 3 — Give the finding in one sentence, without jargon. "He is not behind in his development. He is a boy with a lot of energy and a hearing test that has never been done." One sentence, and then stop.

Step 4 — Normalise, and be specific about what is normal. "What you are describing is very common at five, and the reason it is common is that the part of the brain that stops an impulse is still being built."

Step 5 — Give the one developmental fact they will remember. One. Not three. It must be sayable in a single sentence and repeatable to a spouse that evening.

Step 6 — Check understanding by asking them to say it back. "Say that back to me in your own words, because I want to know whether I said it clearly." This is not a test of the parent; it is a test of the therapist, and it is the step trainees most often skip.

Step 7 — Offer one action, and ask whether it is possible. "Could you do that on the worst evening of the month?" If the answer is no, the action is wrong, and the therapist adjusts until the answer is yes.

Step 8 — Write it on a page, in their words. Not in the therapist's. A page in the parent's own phrasing is followed; a page in clinical language is filed.

Step 9 — Set the review, with the exception named. A date, and the specific things that would bring them back sooner.

Step 10 — Ask the closing question. "What are you going to tell his father tonight?" Or the grandmother, or the aunt. The question surfaces the household's information system, and it frequently reveals that the plan will be contradicted within an hour of the parent getting home.

11.3a Telling a parent something difficult

Some feedback is not normalising. A developmental delay, a probable diagnosis, a safeguarding concern, a recommendation that a child be assessed by another service — these require a different structure, and the difference is that the therapist is delivering news the parent will remember for years.

Before the conversation. Decide what the one piece of news is, in one sentence, and say it aloud twice before the session. Decide what you do not yet know, and be ready to say so. Arrange for both parents to be present if possible, and for someone to be with the parent afterwards. Book a longer slot than usual, and book the follow-up before the conversation begins.

The structure, in six moves.

1. **Signal.** "I want to talk about what the assessment found, and some of it is not what you were hoping for." Two seconds of warning, which is the difference between being told and being ambushed.
2. **The news, in one sentence, without hedging.** "His language is about two years behind where we would expect."
3. **Stop.** Silence. The parent needs time, and anything the therapist says in the next thirty seconds will not be heard.
4. **The meaning, in the parent's terms.** "I know that what you are hearing is that the thing you were afraid of is true." Naming the fear accurately is more useful than reassurance.
5. **What is not changed.** "He is the same boy he was this morning. He is affectionate, he is good with his hands, and he has a mother who noticed." True sentences only.
6. **What happens next, concretely.** A referral, a date, a name, a telephone number, and a follow-up appointment with the therapist. Nothing is left open.

What not to do. Do not deliver the news in the last five minutes of a session. Do not use a diagnostic label without the explanation that goes with it. Do not follow the news with a programme, because the parent will not hear it. Do not say "I am sure it will be fine" — the parent will remember that sentence, and it will be the one that makes them distrust everything else you said.

Afterwards. Write to the parent, briefly, within a week: what was said, what happens next, and the telephone number. A letter after difficult news is one of the few interventions in this book with no downside, and it is almost never done.

11.4 Three techniques

Technique 1 — The one-sentence formulation, said to the parent's face. Write the formulation as one sentence, in plain language, and say it: "your son is not naughty; he is a five-year-old with an undiagnosed hearing problem, and he has been getting into trouble for two years for something he cannot help." The sentence does three things at once: it explains, it removes blame and it implies the action. If the sentence cannot be written in plain language, the formulation is not yet finished.

Technique 2 — The three-week review card. A card with the date, the action, and the two exceptions that would bring the parent back sooner. The card is the treatment's memory, and it survives a fortnight in which the parent is too busy to think about any of it.

Technique 3 — The household rehearsal. Ask the parent to rehearse, in the session, the sentence they will use with the other caregiver. Then ask them what that caregiver will say back, and rehearse the answer. Families do not fail at the clinic; they fail at the front door, when the grandmother says "in my time we did not do that".

11.5 The assessed role-play

The parent-feedback role-play is worth twenty per cent of the module mark. The assessment is deliberately public: each student performs in front of the group for ten minutes, and the group marks against the same rubric the examiner uses, which is the fastest way to teach a skill that is normally learned by watching someone else do it badly for years.

The set-up. A brief of no more than 150 words, containing the age of the child, the presenting concern, one thing the parent has already tried, one thing the parent believes that is wrong, and one live emotion in the room (shame, anger, exhaustion, or fear of a partner's reaction). The student has ten minutes with the parent and no notes.

What is being marked. Not warmth. Not the accuracy of the developmental content in the abstract — the examiners accept any defensible fact. What is marked is the structure in use: did the parent leave with (a) a normalising statement they could repeat, (b) one fact, (c) one action they had agreed to, and (d) a review date with an exception named.

The four ways students fail.

- **The lecture.** Three facts and five recommendations in ten minutes, delivered to a parent who has not been asked a single question. The mark is low irrespective of accuracy, because nothing will be done.
- **The reassurance.** "Do not worry, he will grow out of it", delivered before any assessment has been described. This fails on the rubric's accuracy criterion, and it is the commonest failure among trainees who are naturally warm.
- **The collusion.** Agreeing with the parent's formulation because disagreeing feels unkind ("yes, his father is very strict"). This fails, because a therapist who agrees with one parent against the other has joined the household's conflict.
- **The hedge.** Never stating a finding, so that the parent leaves with no information at all. This is common among trainees who have been taught that they must not diagnose,

and the correction is that describing is not diagnosing: "his language is behind what we would expect, and that needs an assessment" is a description, and it is what the parent came for.

11.5a The parent who arrives with a diagnosis from the internet

The commonest new complication in this work, and it has a structure that can be taught in ten minutes.

What arrives. A parent who has read about a condition — attention deficit, autism, attachment disorder, sensory processing, "highly sensitive child", or a term from a social media video — and who is either relieved to have a name or frightened by it. The parent may ask the therapist to confirm the label, or may resent any suggestion that the label is not the whole picture.

What not to do. Do not dismiss the reading. A parent who has spent three months researching is a parent who is trying, and dismissing the research dismisses the effort. Do not confirm a diagnosis you have not made, which is both a scope error and a clinical one. And do not argue about the label, which converts the session into a debate about a word.

The four-step response.

First, ask what led them to it. "What did you see that made you look that up?" The answer produces the behavioural description the therapist needs, and it is usually better than the label.

Second, separate the description from the name. "The things you are describing — not sitting still, covering his ears, needing the same routine — are real and I saw two of them today. The name is a decision for an assessment, and the assessment is the way to get it right."

Third, say what would change either way. This is the step that resolves most of these conversations: "whatever the assessment says, the things we would do first are the same — protect his sleep, give him one instruction at a time, and find the thing he can do for twenty-five minutes." A parent who discovers that the immediate plan does not depend on the label stops needing the label today.

Fourth, name the referral honestly. If an assessment is indicated, say so and give a date. If it is not, say what the therapist would need to see for it to become indicated, in behavioural terms. A parent who has been told the threshold can watch for it, and watching is itself useful.

The sentence that holds the whole position. "You may be right, and I am not going to pretend to know before we have looked. What I do know is what to do this month." Parents accept uncertainty about the future far more readily than they accept being told that their observation was wrong.

11.6 Case: the mother who has been told she is too soft

Referral. Chidi, seven, in Primary 2, brought by his mother, Adaeze, 33, a tailor, on the school's recommendation. The school's note: "Chidi does not sit still, does not finish his work, and is rough with the smaller children. He is bright. The mother is not firm with him."

The household. Adaeze; Chidi; a daughter, 4; Adaeze's husband, Emeka, 38, a long-distance driver, home four days a month; and Emeka's mother, 61, who lives in the same compound and who believes Chidi is spoiled. Adaeze runs her tailoring business from the front room.

The presenting complaint, in the mother's words. "They say I am too soft. His grandmother says it. His father says it when he comes home and then he shouts at him and it is worse for a week. The teacher says I should beat him. I do not want to beat him. I do not know what to do and I am tired."

What the assessment finds. Vision normal. Hearing never tested. Language: Chidi's vocabulary is good, his sentence construction is age-appropriate, and he cannot follow a three-step instruction in a noisy room — which the therapist tests by giving one, twice, in the consulting room and in the corridor. Attention: he can sustain a preferred activity for twenty-five minutes and a non-preferred one for four. Sleep: he goes to bed at ten, shares a bed with his sister, and wakes when the compound is noisy at five. Diet: he eats once a day, in the evening, because the family eats late. School: forty-eight children in the class, and his seat is at the back.

The five findings.

1. **There is no evidence of a conduct problem and there is evidence of an unassessed hearing and attention picture.** The therapist does not diagnose ADHD; she describes what she saw, names two things that must be excluded, and refers for hearing and for a developmental assessment.
2. **The child is not naughty at four in the afternoon; he is a seven-year-old who has eaten nothing since the previous evening, slept six hours, and is being asked to sit in a class of forty-eight.** The environmental account is not an excuse; it is a set of variables that can be changed.
3. **The mother is being managed by three authorities and supported by none.** The school blames her, the grandmother blames her, the father arrives monthly and shouts. Her "softness" is not a parenting style; it is what remains of a woman with no allies.
4. **The father's monthly arrival is the family's actual behavioural cycle.** Chidi is worse for a week after each return, which the school reads as deterioration and which is better understood as a boy who has been shouted at by a man he sees four days a month.
5. **The most useful intervention in the case has nothing to do with Chidi's behaviour.** It is an alliance with the mother that makes her the author of a plan rather than the object of three plans.

The feedback conversation, in order. The therapist asks what Adaeze thinks is happening. She says: "he is a good boy but he is stubborn, and I am too soft." The therapist asks what she has tried. She lists six things, including two she has invented herself, and the therapist names one: "you have already changed his bedtime once and it worked for a while. That is not a soft mother; that is a mother who experiments."

The one-sentence finding. "Chidi is a bright seven-year-old whose hearing has never been tested and who is being asked to sit still in a class of forty-eight on six hours' sleep and no breakfast, and the behaviour you are being blamed for is the predictable result."

Adaeze cries. The therapist waits. Then: "and you have been told by three people that the problem is you, which is why you are tired."

The four parts, delivered.

Normalise: "Boys of seven who cannot sit still in a hot classroom are extremely common, and it is not a sign that a mother has failed."

The fact: "At seven, a child can hold one instruction in mind but not three, especially in a noisy room. He is not ignoring you; he is losing the middle of the sentence."

The action: one. "Breakfast before he leaves the house, even if it is bread and a boiled egg, and say the instruction one step at a time: 'put your shoes on', then when that is done, 'bring your bag'." The therapist does not give the sleep change, the hearing referral and the school request as actions; those are the plan, and they are on the page, but only one thing is asked for this week.

The review: "Three weeks. If the school reports he is hurting another child, come back at once. If the hearing test comes back abnormal, we will change the whole plan."

The rest of the plan, on the page. A hearing test, booked by the clinic. A letter to the school asking for a seat at the front and one named adult. A developmental assessment referral. A conversation with the grandmother, held by Adaeze, rehearsed in the session, in which she says one sentence: "the doctor says his hearing has never been tested and we are doing that first." And a request to the father, by telephone, in which Adaeze says one sentence she has rehearsed: "when you come home, I need you to ask him about school before you tell him anything."

The grandmother, handled. The therapist does not invite the grandmother to the session, and explains why: the household has three authorities and no hierarchy, and adding a fourth voice — the therapist's — will not produce agreement. Instead Adaeze is given the sentence to say, and the authority to say it, because the grandmother's objection is not to the plan; it is to being excluded from it. The sentence makes her the person who is informed first.

11.6a Six weeks later

The hearing test is normal, which the therapist records as a genuinely useful negative: two years of "not listening" have been excluded as a hearing problem, and the school has been told so in writing.

Chidi is eating breakfast. The three-step instruction has become a two-step instruction, because Adaeze found that three was still too many, and she adjusted it herself. This is the outcome the week is aiming for: a parent who modifies a plan is a parent who owns it.

The school has moved his seat and named a teacher. The rough behaviour has reduced, not disappeared. The developmental assessment has a date in eleven weeks, and the therapist has written to say that the assessment is pending and that the school's "not firm" note should be removed from the file, which it is.

Emeka has been telephoned once by the therapist, at Adaeze's request, for four minutes. He said little. He did, on his next return, ask Chidi about school before saying anything else, and Adaeze reports that Chidi talked for eleven minutes.

The honest limitation. The grandmother's view has not changed. She now says it less in front of Chidi, which is the achievable outcome and is worth having: a child who hears one criticism instead of three is better off even when the third person's opinion is unchanged.

11.7 Transcript: the ten minutes, annotated

The transcript is the assessed role-play's model answer, compressed. The annotations name the part of the structure being used at each turn.

> **T1 Therapist:** Before I tell you anything, I want to know what you think is happening. — Step 1. The single sentence that distinguishes this conversation from a consultation with a verdict. **T2 Adaeze:** He is a good boy. But he is stubborn, and I am too soft. Everybody says so. **T3 Therapist:** Who is everybody? — Three words. The answer identifies the household's authority structure, which is the actual clinical problem, and it takes eight seconds. **T4 Adaeze:** The teacher. His grandmother. His father when he comes. **T5 Therapist:** And what have you tried? — Step 2. The question that finds the parent's competence, which in this case has been invisible to her for two years. **T6 Adaeze:** (lists six things) **T7 Therapist:** You changed his bedtime in January and it worked for a while. That is not a soft mother. That is a mother who experiments. — Naming competence with evidence. Note that the therapist does not say "you are a good mother", which is a verdict the parent cannot use; she describes a behaviour. **T8 Adaeze:** (quietly) Nobody has said that to me. **T9 Therapist:** Here is what I found, in one sentence, and then we will talk about it. (pause) Chidi is a bright seven-year-old whose hearing has never been tested, who is being asked to sit still in a class of forty-eight, on six hours' sleep and no breakfast. The behaviour you are being blamed for is the predictable result. — Step 3, and the sentence is prepared, plain, and contains no diagnosis. It also answers the question underneath, which is "is it my fault?" **T10 Adaeze:** (cries) **T11 Therapist:** (waits, offers the box of tissues, says nothing) — Step 3b. The silence after the finding. Anything said here will be remembered instead of the finding, and the finding is what she needs to remember. **T12 Therapist:** (after about twenty seconds) Say that back to me, in your words, so I know whether I said it clearly. — Step 6, and the pronoun is deliberate: the fault for an unclear sentence is assigned to the therapist, which is what makes the check honest.

11.7a When the complaint comes from the school

A frequent configuration: the referral originates with a school, the parent arrives defensive, and the therapist is positioned between an institution and a family. Three practices keep the therapist from becoming the school's representative.

Practice 1 — Distinguish the observation from the verdict. "Chidi does not finish his work" is an observation. "The mother is not firm with him" is a verdict, and it is a verdict about an adult who is not in the room. The therapist works with the first and declines the second, in writing if necessary.

Practice 2 — Ask the parent what the teacher would say if asked. The answer is often accurate and sympathetic, and it reveals that the parent already knows the school's view and has been carrying it home. Naming that — "you have been told this for two years and you have kept coming back" — is frequently the moment the alliance forms.

Practice 3 — Give the parent something to give the school. A sentence, a request, a letter. A parent who arrives at the school gate with a specific request from a clinic is in a different position from a parent who arrives with an explanation, and the difference is not rhetorical: schools respond to requests with names and dates attached.

The boundary. The therapist does not become the family's advocate against the school, and does not become the school's agent inside the family. The position is with the child's development, which both institutions say they are serving, and which is the only ground on which both can be held to account.

11.8 Laboratory and the marking scheme

The laboratory runs in two parts across two weeks. Part one is rehearsal with peers, unmarked. Part two is the assessed role-play in front of the group.

Marking scheme (20 marks).

Criterion	Marks	What the top band looks like
Structure in use	6	All four parts present and in order, without the parent noticing a structure
Accuracy	4	The developmental content is correct for the age, and no diagnosis is made in the room
One action	4	One action, agreed rather than prescribed, tested against the worst evening of the month, written in the parent's words
The review	3	A date and a named exception, both written
Holding the frame	3	When the parent disagrees, the therapist neither capitulates nor argues; she names the disagreement and offers a review

The pass standard. A student who reassures for ten minutes and gives no action has failed, however warm the room felt. A student who gives five actions has also failed, however accurate the content.

The role-play briefs. Four, rotated. Each contains one emotion and one false belief.

Brief A — the exhausted mother of a two-year-old. She believes the problem is that he "does not respect her". The emotion is shame. The hidden question is "is it my fault?"

Brief B — the father of a nine-year-old who refuses school. He believes the boy is lazy. The emotion is anger, covering fear. The hidden question is "will he turn out like me?"

Brief C — the mother of a fifteen-year-old who has been caught lying. She believes she must search the room. The emotion is fear. The hidden question is "what do I do tonight?"

Brief D — the parents of a four-year-old with a speech delay. They believe he will talk when he is ready, as his uncle did. The emotion is resignation. The hidden question is "is something wrong with my child?"

The debrief question, asked of the group every time. "What did the parent leave with that they did not have when they came in?" The answer must be a sentence, an action and a date. If the answer is "they felt better", the role-play failed, and the group must be able to say why.

11.9 Six errors

Error 1 — Reassuring before assessing. The commonest error in the whole of paediatric practice, and the most damaging, because the reassurance is remembered and the assessment is not. Reassurance after an assessment is a finding; before one, it is a guess with a kind face.

Error 2 — Giving the parent the therapist's formulation. "He has a regulatory difficulty secondary to maternal inconsistency" is a sentence for a file. Said to a parent, it produces shame and no action.

Error 3 — Five recommendations. The parent will implement none and will not say so. One action, chosen for the worst evening of the month.

Error 4 — Agreeing with one parent against the other. "Yes, his father is very strict" feels supportive and is a coalition. The therapist's position is with the household's capacity to change, which requires both parents in the room eventually.

Error 5 — Treating the parent as the patient without saying so. Sometimes the parent is the focus, and when that happens it must be said: "the work I am proposing is with you, not with him, and I want to be clear about that." A parent who discovers halfway through that they have become the client will leave.

Error 6 — Leaving the household's other authorities out of the plan. The grandmother, the aunt, the father on the telephone. A plan that requires a parent to contradict three people alone will fail at the front door, and the therapist will conclude that the parent was not motivated.

11.9a Six sentences that do harm, and what to say instead

Small language choices do more damage in parent feedback than any amount of wrong content. The six below are the ones trainees use most.

"He is doing it for attention." True and useless, because attention is a legitimate need and the sentence implies a fault. Instead: "he has found that this is the way to get you, and we are going to give him a better way."

"He is testing the boundaries." A metaphor that makes the child sound like a strategist. Instead: "he is finding out whether the rule is the same on a bad day, which is what every child does at this age."

"You need to be consistent." An instruction to have a quality, not to do a behaviour, and it is heard as "you are failing." Instead: "pick one rule to hold this week, and let the others go."

"In my experience..." Places the therapist's experience above the parent's knowledge of their own child, and invites a comparison the therapist will win. Instead: "what does he do when you try that?"

"He will grow out of it." A prediction, usually accurate, always remembered, and it prevents the review. Instead: "this pattern usually changes as language develops. I would rather see you in three weeks than guess."

"There is nothing wrong with him." Meant kindly, heard as "your concern is imaginary". Instead: "I did not find the things that would worry me today. What I did find is X, and that is worth working on."

The exercise, in the laboratory. Students are given these six sentences on a card and asked to deliver each one to a partner in role, then to deliver the replacement, and to say which one produced a response. The difference is felt immediately and is remembered for years.

11.10 Culture, faith and talking to parents

The authority to say it. In many households a young clinician, and particularly a young woman, will not be heard on the subject of child-rearing by an older relative. This is not resistance to the content; it is a fact about standing. The workable moves are three: route the information through the person who has standing (a paediatrician, a religious leader, a respected aunt); give the parent a sentence that carries the clinic's authority ("the doctor says his hearing has never been tested"); and never require the parent to win an argument with a grandmother in order to follow the plan.

Corporal punishment. A substantial proportion of the parents in these clinics were beaten and believe that they turned out well, and many schools still recommend it. The therapist who opens with a moral position will end the conversation. The sequence that works is: ask what the parent believes and why; give the developmental fact (what beating does to the behaviour being targeted, over what timescale); give one alternative that is specific and feasible; and accept that the change may be partial. A parent who shouts less is a better outcome than a parent who leaves the clinic believing they have been judged.

Faith and the child's condition. Where a child's difficulty is understood spiritually, the parent may be pursuing prayer and may be advised by a religious authority not to accept a referral. The clinical position is to work with the framework: ask what the prayer is for,

offer to speak with the religious authority if the parent wishes, and hold one non-negotiable — that a hearing test and a developmental assessment are not acts of unbelief. Framing them as seeing what God has made is not a manipulation; it is a translation, and translation is part of the job.

The father on the telephone. In households with labour migration or long-distance work, the father is present by telephone and his authority is exercised remotely. A plan that does not include him will be reversed by him. The four-minute telephone call is a clinical instrument: one sentence from the therapist, one question to him, and one agreement. It is worth more than four sessions with the mother alone.

> **DEBATE**
> "A therapist should not tell a parent what to do." Argue for: advice creates dependence, parents know their children, and the evidence on autonomy support favours collaboration over instruction. Argue against: parents come for advice, a refusal to give it is a withholding of expertise, and a parent with a seven-year-old who has not eaten breakfast needs an instruction rather than a reflective conversation. Then state your rule for when you instruct and when you ask, and give the case that would make you break it.

11.11 Evidence note

What the research supports. The content of the four parts is well supported: psychoeducation about normative development reduces parental anxiety and increases the accuracy of parental attributions; specific, behavioural, positively framed instructions are followed more often than general advice; and brief, structured parenting interventions with a scheduled review produce better outcomes than open-ended support. The finding that parental attributions — whether the parent believes the behaviour is intentional — predict both parental response and treatment outcome is robust and is the empirical basis for the normalising step.

What is weaker. The evidence on how to deliver feedback is thinner than the evidence on what to deliver. The four-part structure is assembled from clinical teaching and from the psychoeducation literature rather than tested as a package. The claim that one action outperforms five is plausible, consistent with the implementation literature, and not directly tested in this form.

What is not supported. That reassurance reduces anxiety when the concern is real — the evidence suggests the opposite, because a parent whose concern is dismissed becomes more anxious and less likely to disclose. That insight alone changes parenting behaviour. And that a single feedback session is sufficient for established difficulties; the evidence favours brief structured contact with review, which is what the fourth part exists to create.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same feedback, given to a mother with three authorities against her and given to a mother with one ally, produces two different families, and the difference is not in the feedback.

11.12 Key terms

Normalising — locating the child's behaviour in a population, to reduce shame and change attribution.

Attribution — the parent's belief about why the child behaves as he does; intentional versus developmental attributions predict different parental responses.

One action — a single, feasible, observable behaviour agreed with the parent.

The exception — the specific change that would bring the family back sooner than the review date.

Household rehearsal — practising the sentence the parent will use with the other caregivers, and their likely reply.

Standing — the social authority to be heard on a given subject, which is not the same as expertise.

Psychoeducation — structured information about a condition or a developmental stage, given to reduce distress and improve management.

Coalition — an alliance between the therapist and one family member against another; the characteristic error of sympathetic practice.

11.13 Self-test

1. Give the four parts of the feedback skill and say what each one prevents.
2. Why is reassurance before assessment an error, and what is the evidence?
3. Name the five questions parents are actually asking, and give one sentence that answers each.
4. What is the test question for choosing an action, and why is it asked rather than assumed?
5. Why does the therapist in the Chidi case not invite the grandmother to the session?
6. What are the four ways students fail the assessed role-play, and what do all four have in common?
7. How would you handle a parent who believes the child's difficulty is spiritual and has been advised not to accept a referral?
8. What is the "exception" in a review, and why is an unconditional review heard as a threat?

Answers. (1) Normalising prevents shame and closes the disclosure problem; the fact prevents blame and changes attribution; the action prevents paralysis; the review prevents abandonment and creates the conditions for the therapist to learn the outcome. (2) Because it is a guess delivered with a kind face and it is remembered instead of the assessment; the evidence is that dismissing a real concern increases anxiety and reduces later disclosure. (3) Is something wrong with my child — "what you describe is common at this age, and the things that would worry me are X, Y and Z, which he does not have"; is it my fault — "a parent asking that question is usually doing more than they think"; will it

get better — "this pattern usually changes as language develops; I would rather review than guess"; what do I do tonight — give the action; am I a bad parent — "you noticed and you came, which is what the parents I worry about do not do". (4) "Could you do this on the worst evening of the month?" — asked because feasibility, not correctness, determines whether an action is carried out. (5) Because the household has three authorities and no hierarchy, and adding the therapist's voice produces a fourth rather than an agreement; the sentence given to the mother makes the grandmother the person informed first, which addresses the actual objection. (6) The lecture, the reassurance, the collusion, the hedge — all four end the conversation rather than opening it. (7) Work inside the framework: ask what the prayer is for, offer to speak to the religious authority, and hold one non-negotiable — that a hearing test and a developmental assessment are not acts of unbelief, since they are ways of seeing what has been made. (8) The specific change that would bring the family back sooner; without it the review is heard as "come back when you have failed again".

11.14 Reflective task

Record yourself giving feedback to a colleague in role, and listen to it twice. On the first listening, count the number of recommendations you gave. On the second, listen for the moment your voice changed when the colleague disagreed with you. Most trainees find between three and six recommendations and a voice that either rose or softened into agreement. Both are the same failure: the therapist leaving the frame in order to keep the room.

11.15 Further reading

- Hattie, J. and Timperley, H. "The power of feedback" — the general evidence on what makes feedback usable.
- Sanders, M. Positive Parenting Program materials — the worked examples of specific, behavioural instructions.
- Johnston, C. and Mash, E. "Parental attributions" — the empirical basis for the normalising step.
- Brazelton, T. Touchpoints — the classic account of developmental information given to parents well.
- Buckman, R. Breaking Bad News — the six-step protocol adapted in 11.3a.

Next: Week 12 — Later life: integrity, loss and caregiving.

Week 12 — Later life: integrity, loss and caregiving

Developmental tasks. Integrity versus despair (Erikson, from about sixty): the consolidation of a life into something that can be told as a whole, or the conviction that it did not add up. Adaptation to retirement, which is a role loss before it is a financial one. Bereavement, which at this stage arrives repeatedly and in sequence. Caregiving, which reverses the generational direction. Grandparenthood, which is the longest single role in most people's lives and the least theorised. And legacy — what is handed on, and to whom.

Clinical realities. Cognitive change must be distinguished from delirium, depression and dementia, and from hearing loss — which is a major and under-treated contributor to isolation and apparent confusion. In this age band more mistaken diagnoses are made by omission of a hearing test than by any other single error. Caregiving burden falls disproportionately on women and on the "responsible" child, and it frequently reopens sibling hierarchies that were settled forty years ago.

Grief. Use the dual-process model (Figure 22, MFT 712): healthy grieving oscillates between loss-oriented and restoration-oriented activity, and the oscillation is the mechanism, not a sign of avoidance. In many cultures, mourning is collective and prescribed; the clinician's task is to support the rites rather than to impose a private model of "working through". A widow who is surrounded by prescribed mourning for forty days and then alone is not in a pathological grief; she is in a transition that the rites mark and the culture does not follow through.

Why this week is difficult to teach. Because it concerns the end of the story, and because students carry their own grandparents into the room. The week is also the most practically useful in the second half of the course: families with an ageing parent make up a large share of the referrals a family therapist receives, and the work is concrete — a rota, a hearing test, a family meeting — rather than interpretive.

> **CASE**
> Three siblings disagree about their mother's care. The eldest, a daughter, has done everything — the medical appointments, the money, the telephone calls — from another city. The son with whom the mother lives has a wife who does the daily work and who has not been consulted. The youngest son lives twenty minutes away and visits twice a year. The referral comes from the daughter-in-law, and she is the only person in the family who is not related to the patient.

12.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The last stage	Why integrity is a task and not a mood	Name one older person whose life they would call "integrated", and say why
8–20	Retirement	The role loss, the two sexes' different experiences, and the somatic presentation	Predict the presentation for three retirees
20–34	The cognitive differential	Delirium, depression, dementia, hearing — the four-way table, worked	Sort eight observations into the four categories
34–46	Caregiving	Burden, the "responsible child", and the reopening of sibling hierarchies	Map a caregiving network and its unstated contract
46–58	Grief	The dual-process model, and why oscillation is health	Classify six bereavement behaviours
58–70	The family meeting	Structure, roles, and the one thing that must be decided	Run a five-minute family meeting in role

Minutes	Segment	What the lecturer does	What students do
70–82	Grandparenthood and legacy	What is handed on, and to whom, in households with distributed care	Interview a peer about what they received
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students arrive with a sentimental model of old age — wisdom, grandchildren, a rocking chair — or a medical model — decline, burden, placement. Both are ways of not asking. The lecture's job is to install a practical attitude: what does this person need, who is providing it, who is not being consulted, and what would have to change this week? The demonstration is the caregiving network map, drawn in five minutes on the board, because it makes visible in one diagram the eleven people who are involved and the one who is doing the work.

The sentence the lecture must land. "The question in later life is almost never 'what does this person need?' — everybody knows that. It is 'who is going to do it, and who has been deciding without them?'"

12.2 Five concepts that organise the week

1. Integrity versus despair. Integrity is the capacity to hold one's life as a whole — including its failures — and to accept it as having been worth living. It is not contentment and it is not wisdom. It is an achievement of narrative, and it is made possible or impossible by the audience available. A seventy-eight-year-old with grandchildren who ask questions has a different task from one whose contemporaries have all died and whose children telephone on Sundays.

The clinical translation: the therapist's task is not to supply meaning but to elicit the account. The life review (below) is the technique, and the evidence for it is modest but consistent: structured reminiscence reduces depressive symptoms in older adults, and its mechanism appears to be the construction of coherence rather than the discharge of emotion.

2. Retirement is a role loss before it is a financial one. The research finding that matters is that the loss of the work role predicts distress more strongly than the loss of income, and that the effect is largest for men whose identity was occupational and for people whose retirement was involuntary. The presentation in a family clinic is usually somatic — fatigue, pain, a "heart" that has been investigated — and the question that opens it is "what were you supposed to be doing now?"

3. Bereavement in later life arrives in sequence. The distinguishing feature of grief at this stage is not intensity but accumulation: the death of a spouse is frequently preceded by siblings, friends, and sometimes a child. The dual-process model is useful here because it normalises the oscillation — a widow who laughs at a funeral and weeps while sorting a wardrobe is not confused; she is moving between loss-oriented and restoration-oriented coping, and both are work.

4. Caregiving is a role with no induction. The caregiver is usually appointed by proximity, gender, or birth order, rarely by negotiation, and the appointment is rarely

spoken. The clinical consequences are three: the caregiver's exhaustion is invisible because it is expected; the caregiver's resentment is unmentionable because it feels like a betrayal; and the person being cared for is frequently not consulted at all, which is the dignity injury that the family never notices.

5. Grandparenthood and legacy. In households with distributed caregiving, the grandparent is often a primary caregiver rather than a visitor, and the role runs in both directions: the grandmother raising children while the parents work, and later the adult children caring for the grandmother. The legacy question — what is handed on, and to whom — is answered by what the older person is actually asked. A grandmother whose only conversations are about medication has no route to give anything, and the giving is the developmental task.

12.2a The four-way cognitive differential

This table is the most practically important page in the week. In an older person with confusion, four conditions account for the great majority of presentations, and three of them are reversible or treatable.

Feature	Delirium	Depression	Dementia	Hearing loss
Onset	Hours to days	Weeks to months	Months to years	Gradual, often years
Course	Fluctuates through the day	Persistent, worse in the morning	Progressive, steady	Consistent; worse in noise
Attention	Impaired, wanders	Intact but poorly sustained	Intact early, impaired late	Intact
Mood	Fearful, agitated	Low, hopeless, anhedonic	Variable; may be apathetic	Often withdrawn, embarrassed
Memory	Impaired, patchy	Complains of memory, performs better than complaint	Impaired, especially recent	Appears impaired because of mishearing
Reversibility	Usually, if the cause is treated	Usually	Not currently	Correctable with aids
The question that reveals it	"Was he like this yesterday?"	"When did you last enjoy something?"	"When did you last manage the money on your own?"	"Does he answer better when he can see your face?"

The three rules that follow.

Rule 1 — Any sudden change in an older person is delirium until proved otherwise, and it is a medical emergency. A urinary tract infection, constipation, dehydration, a new medication, and pain are the common causes, and all are treatable. A family that has been told "it is dementia" after a two-day change has been told something that may be false and that will organise the next five years.

Rule 2 — Depression in later life looks like dementia and is more treatable. The label "pseudodementia" is unhelpful; the phenomenon is real. The depressed older person

answers "I do not know" to cognitive questions, while the person with dementia attempts an answer and gets it wrong. That single observation distinguishes them better than any test the therapist can administer.

Rule 3 — Test the hearing before interpreting the confusion. The commonest scene in this week's work is an older woman described as "difficult" and "not following what is said", who is simply not hearing. The therapist's test is free: sit facing her, at her level, in a quiet room, and repeat the same question with and without your face visible. A difference between the two conditions is the diagnosis.

12.2b Caregiving: the network and the unstated contract

The caregiving network map is drawn in six steps and takes about eight minutes on a whiteboard. It is the week's assessment instrument and, in most cases, the intervention as well.

Step 1 — Put the older person in the centre. Name, age, and the one sentence that describes what she can and cannot do. Not a diagnosis; a function. "She can wash and dress herself. She cannot manage her medication, leave the house, or remember what she was told an hour ago."

Step 2 — Draw everyone who does something, in a ring. Include the daughter-in-law. Include the neighbour who looks in. Include the paid carer, if there is one. The map routinely reveals that the "family" doing the care is five people, of whom two are not related.

Step 3 — Write, next to each name, what they actually do, in hours per week. The numbers are estimates and the estimates are the finding: the person who does forty hours is usually the one who was never asked, and the person who does two is usually the one whose opinion carries the most weight.

Step 4 — Mark who decides. A different marker, on the same names. The gap between who does the work and who makes the decisions is the network's fault line, and it is the source of most of the conflict that arrives in the clinic.

Step 5 — Mark who is not consulted. The person in the centre is almost always on this list, and so is the daughter-in-law. Writing "not consulted" next to the patient's own name is the single most useful intervention in the exercise, because it makes visible an omission that no one in the family has noticed.

Step 6 — Ask the network question. "If the person doing the most stopped tomorrow, what would happen?" The answer is the network's fragility, and it is usually discovered in the same session in which the family arrived complaining about something else.

The unstated contract. Every caregiving network has one, and it has four clauses: who is responsible; what counts as good care; what the carer is entitled to; and what happens if the carer refuses. None of the four is ever spoken, and each is a source of conflict when it is breached.

The clinical move is to make the contract speakable without making it an accusation. The four questions: "who decided that you would be the one? what would count as doing this

well? what are you allowed to have for yourself? and what would happen if you said no?" The fourth question is the one that produces tears, because most carers have never considered that refusal is an option and have been living as though it is not.

The burden measures, used carefully. Standardised burden instruments (Zarit, and similar) are useful for research and for demonstrating severity to a family that has normalised it. In the room, the two questions that do most of the work are: "when did you last sleep through the night?" and "who asks how you are?" The second question identifies the carer's isolation more reliably than any scale.

12.2c Retirement: the presentation, and what to ask

Retirement is the developmental event most often missed in a family clinic, because it presents as something else.

The four presentations. A somatic complaint that has been investigated and explained as "nothing serious"; a sudden increase in drinking; a man who is under the family's feet and who has begun to manage the household in a way that produces marital conflict; and a withdrawal into a chair that the family describes as depression. All four are the same role loss in different costumes.

What has actually been lost. Not the money, usually. The structure of the day; the colleagues, who were the social network; the competence, which was visible and daily; and the audience, which is what an occupational identity needs. A man who was the branch manager for eleven years does not miss the money as much as he misses being the person whose judgement was sought.

The four questions.

1. "What time did you get up before you retired, and what time do you get up now?" The answer is the structure, quantified.
2. "Who did you talk to every day at work, and who do you talk to every day now?" The answer is the network, quantified.
3. "What are you good at that nobody asks you about any more?" The answer is the competence, and the question is the first hinge of the intervention.
4. "What would you have done with the time if you had been allowed to choose, at fifty?" The answer is frequently a lost project, and the lost project is often still available.

The intervention, in two moves. Restore a structure — a fixed weekly commitment with other people in it, which is what the workplace was providing — and restore a use of competence — tutoring, a committee, a trade association, a church role, coaching, a small business. The evidence on volunteering and structured activity in retirement supports the first more strongly than the second; the clinical experience is that both are necessary and that neither is achieved by advice.

The wife's experience. The retirement literature under-reports the marital consequence: a woman who has run a household for thirty years and whose husband is now present in it for all of it. The clinical presentation is a couple in their sixties who have "started arguing", which is usually a pair of people negotiating territory for the first time since

1994. The intervention is not conflict resolution; it is a division of the day, written, in the way that Week 10's division-of-labour audit is written.

12.3 The method: the family meeting about an older parent

One session, three to six participants, ninety minutes if possible. The structure below is adapted from family meetings in medical settings and is used unchanged in this book.

Step 1 — Establish who is in the room and who is not, and why. Name every absent person and the reason. Then ask whether anyone wants them present. A meeting that excludes the person who pays, or the person who does the daily work, will produce a decision that is reversed within a fortnight.

Step 2 — Ask each person, in turn, one question: "what is your greatest worry?" In turn, without interruption, and written down verbatim. This single round does more than any other part of the meeting, because it converts a group of people who have been arguing about arrangements into a group of people who have each stated a fear.

Step 3 — Put the facts on one page. What the older person can do; what the medical picture is; what the options are; what each costs, in money and in hours. Facts first, feelings second, is the order that prevents the meeting from becoming a re-run of a forty-year-old argument.

Step 4 — Name the unstated contract, out loud. "It seems that the arrangement has been that Bola manages the medical care, Funke does the daily work, and nobody has asked Mama what she wants. Is that right?" The family corrects, and the correction is the working formulation.

Step 5 — Generate four options, not two. The binary — she stays where she is, or she goes into a home — is what has made the argument unwinnable. Four options, written on the page, each with costs. The purpose is not to choose in the meeting; it is to demonstrate that there is more than one way for this to go.

Step 6 — Decide one thing, and only one. A review date, a rota for one week, a hearing test, a conversation with the older person. One decision, made in the room, with a named person and a date.

Step 7 — Say what happens if it fails. The fallback, named in advance. Families that have a fallback implement the plan; families without one abandon it at the first difficulty and conclude that the meeting was pointless.

The older person's presence. If she can participate, she participates. The most common and most damaging omission in these meetings is the exclusion of the person whose life is being arranged, on the grounds that she will be upset or confused. A hearing-impaired woman who is spoken over for ninety minutes will be described afterwards as "difficult", and the description will be accurate.

12.3a When the siblings disagree

Sibling conflict about an older parent is not usually about the parent. It is usually about a ledger that was opened in childhood, and the clinician's task is to keep the present decision from being decided by the old ledger — while acknowledging that the ledger is real.

The four patterns, and the move for each.

The responsible child and the absent ones. One sibling has done everything and is exhausted and resentful; the others have done nothing and feel judged. The absent siblings' position is usually defended with a practical argument ("I am in Lagos", "I have small children"), which is true and is not the whole truth. The move: give the absent siblings a specific, bounded, non-negotiable task, and give the responsible child a specific relief. General appeals to help more produce nothing; "Kunle will take Mama to the clinic on the second Saturday of every month" produces a decision.

The child who is not a child. A daughter-in-law, a nephew, a foster kin who has done the work and has no standing in the family's hierarchy. The move: give them standing explicitly, in the meeting, by having the family agree to their participation. A therapist who fails to do this will watch the most competent person in the room be overruled by the least involved.

The sibling who wants placement and the sibling who refuses it. Usually the difference is about money, guilt, or a promise made to the parent. The move: separate the promise from the plan. "What did she ask you to do?" is asked of each sibling, and the answers are frequently different, which is the finding.

The sibling who is not speaking to another. A silence of years, brought into the room by the parent's decline. The move: do not attempt reconciliation. Ask only whether the two of them can agree on one practical matter for six weeks, on the understanding that nothing else is being resolved. Concrete cooperation across an unresolved conflict is achievable and reconciliation is not, and a therapist who aims at the second will lose the first.

The rule that governs all four. The parent's needs are the organising question and are not the whole conversation. A therapist who insists that "this is about Mama" will be met with agreement and no change, because the siblings know it is not only about Mama, and so does the therapist.

12.4 Three techniques

Technique 1 — The life review, in six questions. Not a memoir. Six questions, asked across two or three sessions, recorded if the older person agrees, and given to the family as a transcript.

1. "What is the first thing you remember?"
2. "What did you think your life would be, and what happened instead?"
3. "Who did you love, and who loved you?"
4. "What are you proudest of that nobody knows about?"

5. "What do you want the ones coming after you to know?"
6. "What would you do differently?"

The technique works because the older person is being asked as an author rather than as a patient, and because each question has a developmental function: memory for continuity, the timetable question for integrity, the love question for relationship, the pride question for self-authorship, the legacy question for generativity, and the counterfactual for the acceptance of a life that was not perfect.

Technique 2 — The legacy recording. A telephone recording of the six answers, given to a grandchild. The instruction to the family is what makes it work: "do not listen to it while she is alive". Its function is not comfort in the present; it is the presence of a voice after the death, which is the thing that grandchildren ask for and that families almost never make.

Technique 3 — The rota, written and limited. A weekly schedule with names beside tasks: who takes her to the clinic, who calls on Tuesday, who sleeps in the house on Friday. Bounded in time — six weeks — and specific in task. The rota does three things that conversation cannot: it distributes the mental load, it makes absence visible, and it converts an obligation into a schedule that can be renegotiated.

The caregiving session, in practice. The therapist who does all three in one session will fail. The sequence that works: the map first (assessment), then one technique (intervention), then the rota (a concrete output). Sessions two and three can add the review.

12.5 The first session with three siblings and a daughter-in-law

Phase 1 — The round of worries (20 minutes). Each person, in turn, one question: "what is your greatest worry?" Written down verbatim. No interruption, no response, and the therapist explains why: "everybody will get a chance to answer the others, and I want the worries on the table first."

Phase 2 — The facts (15 minutes). What Mama can do; what the medical picture is; what the money is; what has been tried. The therapist supplies what she has and asks for the rest, and writes it on one page.

Phase 3 — The map (10 minutes). Drawn on the whiteboard, with hours and decisions marked, and "not consulted" written next to the names that belong there — including, usually, Mama's.

Phase 4 — The contract named (15 minutes). Who decided; what counts as good care; what the carer is entitled to; what happens if she refuses. The four questions, asked slowly, to the group rather than to an individual.

Phase 5 — Four options (15 minutes). On the page, with costs. The therapist's discipline is not to recommend; the family's job is to eliminate.

Phase 6 — One decision and one fallback (10 minutes). Named, dated, written, photographed.

Phase 7 — The absent person (5 minutes). "Who is not here, and who will tell them?" The question is asked at the end, because the answer determines whether the decision survives the week.

Why Mama is asked first, if she is present. Because a meeting about an older person that begins with her children describing her decline has already decided what she is. The therapist's first question to her is not about her health but about her preference: "what would you like the next year to look like?" The answer is frequently surprising, frequently practical, and it reorganises the meeting.

12.6 Case: the Baloguns and the woman in the back room

The family. This household first appeared in MFT 702, where Mr Femi Balogun (now 55) was a long-distance lorry driver away three weeks in four, his wife Funke (49) ran a food business from the house, and his mother (then 78, now 81) was living with them with a diagnosis of dementia. The family has changed since then: the eldest son has left school and is working; the second child, with sickle cell disease, is now 17 and stable; there are two younger children.

The referral. From Funke, the daughter-in-law, by telephone, in tears. "I cannot do this any more. His sister in Lagos decides everything and she is not here. His brother lives twenty minutes away and comes at Christmas. I am the one who bathes her."

The household, as mapped. Mama, 81. Femi, 55, away three weeks in four. Funke, 49, the daily carer. Bola, 58, Femi's elder sister, in Lagos, a retired nurse, who pays for medication and manages appointments by telephone. Kunle, 49, the youngest, in Ibadan, twenty minutes away, visits at Christmas and Easter. Four children in the house, aged 22, 17, 12 and 9. A neighbour, Mama Seri, who looks in on weekdays. A physician at a private hospital who has seen Mama twice in two years.

The hours, estimated in the session. Funke 38 hours a week; Bola 4 (telephone and payments); Kunle 2 (a year); Mama Seri 3; Femi 6 in the week he is home.

The decisions, marked. Bola makes the medical decisions. Femi makes the financial decisions when he is home. Funke makes every decision in between and is not consulted about any of the ones the others make. Mama makes none.

The facts. The dementia diagnosis was made two years ago after a gradual decline. Three months ago there was a sudden worsening over two days — agitation, not recognising Funke, wandering at night — and a locum doctor at the hospital said "the dementia is progressing" and gave a sedative. The worsening resolved in five days. The therapist writes this on the page and circles it, because it is the case's most important fact.

The five findings.

1. The three-month-old "progression" was probably a delirium, and it has never been assessed. A change over two days with agitation and nocturnal worsening, resolving in five days, is the signature of a delirium — most likely a urinary tract infection, constipation, dehydration, or the sedative itself. A diagnosis of progression was made on

the basis of a five-day event, and the family has organised three months of decisions around it. The first action of the case is a medical review, not a family decision.

2. The hearing has never been tested and is the likely source of "difficulty". In the session Mama is spoken to from across the room and does not respond; spoken to from a metre away, facing her, she answers appropriately three times out of four. Funke reports that Mama "is better in the morning" — which is when the house is quiet. The therapist asks for a hearing assessment and, in the meantime, demonstrates the difference in the room, in front of the family. Bola, who is a retired nurse, is visibly affected.

3. Funke is doing the work, has no standing, and is not related to the patient. She is the network's most competent member and its least consulted. Every decision about Mama is made by people who are not there, and Funke's experience of the last three months is that she is a servant with clinical responsibilities. Her tears on the telephone are not a symptom of the family's difficulty; they are the most accurate description of it.

4. Bola's control is functional, not merely controlling. She is a retired nurse, she pays, and she has been managing a mother at a distance for two years with the telephone as her only instrument. Her insistence on deciding is what a frightened eldest daughter does when she cannot be present. The therapist who treats her as the villain will lose the person who pays for the medication.

5. Mama has not been asked anything in two years. Not what she wants, not how she feels, not whether she is frightened. The life review questions have never been put to her, and the family has no account of her life that is longer than the last two years.

The formulation. An eighty-one-year-old woman with a dementia diagnosis and an untested hearing loss, whose recent "progression" may have been a treatable delirium, cared for by a daughter-in-law with no standing in the family's hierarchy, managed medically by a daughter at a distance, and financially by a son who is away three weeks in four. The presenting complaint is Funke's exhaustion, and the family's problem is a caregiving network whose workload and authority are inversely distributed.

The intervention, in seven parts, in order.

Part 1 — The medical review, first. A written request to the physician for a review of the three-month-old episode, with the specific question: was this a delirium, and was the sedative appropriate? The therapist does not diagnose; she puts the question on paper and asks the family to take it.

Part 2 — The hearing test, and the demonstration in the room. The demonstration has already changed Bola's position, and the test is booked for the following week.

Part 3 — Funke gets standing, explicitly, in the meeting. The therapist asks Bola and Kunle, in front of Funke, to agree that Funke is the person who knows Mama's daily condition and that decisions about the daily arrangements will be made with her. This is a single sentence and it is the intervention on which the rest depends.

Part 4 — Bola gets a defined role and a bounded one. Medical decisions and payment, with a monthly telephone call to Funke at a stated time. The therapist names the

reason plainly: "she has been managing from Lagos with a telephone, which is the hardest way to do it, and the answer is not to remove her authority but to give her one that has a schedule."

Part 5 — Kunle gets one task. The second Saturday of every month, the clinic visit, with a written report to the family group. A specific, bounded, non-negotiable task, which he accepts in the room after some resistance and which he performs for eight consecutive months.

Part 6 — The rota, for six weeks. Written, photographed, with a review date. Bathing, nights, medication, clinic, the neighbour's days.

Part 7 — Mama is asked. In a separate session, with Funke present and Bola on speakerphone, the six life review questions are begun. Mama answers three, is tired, and stops. The therapist asks whether she would like to continue next week, and she says yes. The transcript of that session is, six months later, the thing the family values most.

12.6a Six months later

The medical review found a urinary tract infection at the time of the episode and no evidence of progression beyond the expected; the sedative was stopped. Mama's functioning is close to what it was before the episode, which means that three months of family decisions were made on a false premise.

Hearing aids were prescribed and are worn, inconsistently. When they are worn, Funke reports that Mama is "a different person", which is the phrase families always use and which is a description of the difference between a woman who cannot hear and a woman who is confused.

Kunle has done eight clinic visits out of six months, which is not the plan and is more than the previous decade. Bola telephones Funke on the first Sunday of the month. The two women have begun to speak to each other as colleagues rather than as a manager and an employee, which Funke describes as "she asks me now".

The rota held for five weeks and then dissolved. The therapist notes this without alarm: a rota's function in most families is to make the distribution visible once, not to run indefinitely, and the visibility persisted after the document was lost.

The honest limitation. Mama is not going to recover, Funke is still doing thirty hours a week, and the family is still one woman's illness away from crisis. What changed is that the work is now visible, the worker has standing, and the person in the centre has been asked what she wants. Those are the achievable outcomes of this week's work, and a therapist who promises more will be lying to a family that has enough disappointment already.

What would have changed the plan. If the medical review had confirmed progression, the meeting would have become a placement discussion and the four options would have been about care settings. If Bola had refused Funke's standing, the work would have proceeded with Funke alone on the question of what she is entitled to, which is a legitimate treatment in its own right. And if Mama had been unable to participate at all,

the life review would have become a review with the family about her, which is a poor substitute and better than nothing.

12.7 Transcript: asking an eighty-one-year-old what she wants

Ten turns from the session in Part 7, with Funke present and Bola on speakerphone. Mama wears a hearing aid for the first time in the therapist's presence.

> **T1 Therapist:** (facing her, at her level, unhurried) Mama, I want to ask you something that nobody has asked you. (pause) What would you like the next year to look like? — The question is put to her first, in the first person, seated at her level and facing her. Everything about the physical arrangement is the intervention. **T2 Mama:** (long pause) Nobody has asked me that. **T3 Therapist:** I know. That is why I am asking. **T4 Mama:** (pause) I would like to sit outside in the morning. (pause) In the compound. With the sun. — The answer is concrete, small, and entirely outside the family's frame of reference, which is what these answers are almost always like. **T5 Funke:** (quietly) She cannot go outside. There are steps. — The daughter-in-law's objection is practical and true, and it is the beginning of the intervention rather than a contradiction of it. **T6 Therapist:** (to Funke) Then that is the thing we are going to solve. (to Mama) Mama, if someone carried the chair out for you, would you want to sit there? **T7 Mama:** (nodding) In the morning. Before it is hot. **T8 Therapist:** (to Bola, on the speakerphone) Bola, did you hear that? — The question is asked of the absent authority, deliberately, because the decision will not survive unless she has heard it in Mama's voice. **T9 Bola:** (after a pause) I heard. **T10 Therapist:** (to the room) Then the first item on the rota is not the clinic and it is not the medication. It is the chair, in the morning, before it is hot. — The therapist writes it first on the page, above the medical items, which is a statement about what this meeting is for.

12.8 Laboratory: the caregiving network map

Aim. To map a caregiving network in fifteen minutes, identify the inverse distribution of work and authority, and produce one decision that the family can make in the room.

Procedure. Groups of five: therapist, older person, primary carer, distant decision-maker, and one absent sibling represented by an empty chair (which students find unexpectedly powerful). The trainer supplies a one-page brief with hours, decisions and one unassessed medical event. Twenty-five minutes, then feedback.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The map	Verbal description only	Drawn, but without hours or decisions	Drawn with hours, decision markers, and "not consulted" named
The medical check	Accepted the diagnosis on the file	Asked about the diagnosis	Identified the unassessed event and wrote the question for the physician
The hearing	Not considered	Asked about it	Tested it in the room and demonstrated the difference

Criterion	0–1	2–3	4–5
Standing	Left the carer without authority	Acknowledged the carer	Gave the carer standing explicitly, in front of the others
The decision	None	One decision, no date	One decision, dated, named, with a fallback

The distinction that carries most of the marks. The top band requires the student to ask the older person a question and write the answer down. Groups reliably spend twenty minutes on the network and never speak to the patient, which reproduces in the laboratory the exact error the week is about.

12.9 Six errors

Error 1 — Accepting "dementia" as an explanation for a change. Any sudden change is delirium until proved otherwise, and it is a medical emergency. The error is common because the diagnosis is already on the file and because families present the change as deterioration.

Error 2 — Interpreting confusion without testing hearing. The cheapest test in this book and the most often skipped. A hearing loss produces social withdrawal, apparent paranoia, non-compliance and "difficulty", and every one of those is then treated as a symptom of dementia.

Error 3 — Meeting the family without the patient. The most common structural error in later-life work. A woman who is spoken over for ninety minutes will be described as difficult afterwards, and the description will be accurate.

Error 4 — Treating the daughter-in-law as a helper. She is frequently the most competent and least consulted member of the network, and a therapist who addresses the adult children and uses the carer as a source of information has reproduced the family's hierarchy rather than intervening in it.

Error 5 — Aiming at sibling reconciliation. The conflict is usually forty years old and is not the therapist's business. The aim is six weeks of concrete cooperation, which is achievable, and a therapist who aims at the former will lose the latter.

Error 6 — A plan with no fallback. Families implement plans that have a stated fallback and abandon plans that do not. The fallback is not pessimism; it is the condition under which the first plan survives its first bad week.

12.9a The phone call that is really a referral

A pattern worth naming separately, because it is how most later-life work actually begins.

The call comes from an adult daughter, usually in the evening, and it is framed as a request for information: "I just want to know whether we should be worried." Within three minutes the real request appears, and it is one of four.

"Tell me I am doing enough." The caller is asking for a verdict on her caregiving. The answer that works is behavioural rather than reassuring: "you are the one who

telephoned. What are the three things you actually do each week?" Listing them is the intervention, because the caller has never counted.

"Tell me my brother should help." The caller wants an ally. The answer that avoids the trap: "I will not tell him that. I will ask him, in a meeting, what he can do — and I will ask you the same question." A therapist who supplies the ally is then blamed when the brother does not change.

"Tell me it is time for a home." The caller wants permission. Permission is not the therapist's to give, and the honest answer is the structure of the decision: "that is a decision for your family, and I can help you make it in a way you can all live with. What would have to be true for you to consider it?"

"Tell me she is not dying." The caller wants a prediction. Give the medical picture as far as it is known, name what is not known, and offer the one thing that can be offered: a plan for the next month rather than a prognosis for the next year.

The practical point. The first telephone call is the assessment and the engagement, and it is usually taken by the least experienced person in the service. The four framings above should be taught to whoever answers the telephone, because a call that is handled as a request for information and ends with "make an appointment" has lost the family's most motivated member.

12.10 Culture, faith and later life

Who cares for the elderly, and who is expected to. In most of the households this book is written for, care is a family obligation rather than a purchased service, and the obligation falls on a daughter-in-law or a daughter rather than on a son. The clinical consequence is that the person doing the work frequently has no authority, and the person with authority frequently does none of the work. This is the week's central finding, and it is a structural fact rather than a family pathology.

Respect and the impossibility of asking. In many households it is improper for a younger person to ask an older one what she wants, and the impropriety is felt by the family as a failure of respect rather than as an omission. The therapist can use the frame rather than fight it: "it is because we respect her that we are asking her" is a sentence that permits the question. Where the older person cannot be asked, the therapist asks the family to describe what she would say, which is a second-best procedure and is better than nothing.

Placement and its meanings. A care home carries, in many settings, the meaning of abandonment, and a family that is offered placement as an option may hear an accusation. The therapist's discipline is to keep four options on the page without recommending any, because the family's choice must be one the family can live with in front of its relatives, not one the therapist would make.

Faith, illness and the meaning of decline. Households may understand dementia as a spiritual trial, a punishment, or a season, and the understanding organises how much medical investigation is pursued. The clinical position: ask what the framework means for the family, and hold one non-negotiable — that a treatable infection should be treated.

Framing the medical review as stewardship rather than as doubt is translation, not manipulation.

Mourning as a collective practice. Where grief is prescribed — forty days of visits, a year of black, a particular liturgy — the therapist's task is to support the rites and to watch for the period after them, when the company leaves and the bereavement becomes private. The dual-process model describes an individual oscillation; the culture supplies a collective one, and the clinical question is what happens when the collective one ends.

> **DEBATE**
> "The person who does the caring should make the decisions." Argue for: authority should follow knowledge and work, the distant decision-maker is making choices about a life they do not see, and the current arrangement is exploitative. Argue against: decisions about money require the person who has the money, medical decisions require medical knowledge, and a daughter-in-law with authority and no kinship has no standing to defend it against the family. Then state what you would actually have done in the Balogun meeting, and what you would have done if Bola had refused.

12.11 Evidence note

What the research supports. Structured life review and reminiscence reduce depressive symptoms in older adults, with modest but consistent effects. Caregiver burden interventions that combine psychoeducation with practical respite produce better outcomes than either alone, and the effect is larger when the carer is given specific tasks rather than general support. Hearing loss is independently associated with cognitive decline, social isolation and depression, and correction improves social participation. Delirium is common, frequently missed, and associated with poor outcomes, particularly when it is misattributed to dementia.

What is weaker. The evidence on family meetings is largely from medical and palliative settings and transfers imperfectly to a therapy clinic. The dual-process model is well supported descriptively and less well supported as a treatment target. Effects of grandparent involvement on child outcomes are confounded by selection and are difficult to interpret.

What is not supported. That "working through" grief requires the discharge of emotion — the evidence favours oscillation and continued bonds over severance. That cognitive decline is inevitable and untreatable in every case, which is the assumption behind the most damaging errors in this week's work. And that carers who are not relatives are less appropriate; the evidence on non-kin caregiving is thin and the clinical experience is that they are frequently the most reliable member of the network.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same dementia diagnosis produces, in one family, a coordinated network and, in another, a daughter-in-law doing forty hours a week in silence, and the difference is not in the diagnosis.

12.12 Key terms

Integrity versus despair — Erikson's final stage; the consolidation of a life into an account that can be held.

Delirium — an acute, fluctuating disturbance of attention and awareness, usually caused by a treatable physical problem; a medical emergency.

Pseudodementia — the presentation of depression as apparent cognitive decline; treatable.

Dual-process model — the account of grief as oscillation between loss-oriented and restoration-oriented coping.

Caregiver burden — the objective and subjective cost of caregiving; measured, and frequently normalised by the family.

Unstated contract — the four unspoken clauses of a caregiving arrangement: responsibility, standard, entitlement, and the consequence of refusal.

Responsible child — the sibling appointed by proximity, gender or birth order, rarely by negotiation.

Life review — a structured reminiscence intervention aimed at coherence rather than discharge.

Legacy — what is handed on, and to whom; the developmental task that requires an audience.

Fallback — the pre-agreed response if the plan fails; the condition under which plans survive.

12.13 Self-test

1. Give the four-way cognitive differential and the one question that best distinguishes each pair.
2. Why is a change over two days a medical emergency, and what has usually happened when a family is told that "the dementia is progressing"?
3. What is the inverse distribution, and why does it produce the referral?
4. Name the four clauses of the unstated caregiving contract.
5. Why does the therapist write the chair above the medical items on the rota?
6. What are the four sibling patterns, and what is the move for the daughter-in-law?
7. Why is sibling reconciliation the wrong aim, and what is the right one?
8. In the Balogun case, what was the single most important fact, and how was it found?

Answers. (1) Delirium — "was he like this yesterday?"; depression — "when did you last enjoy something?"; dementia — "when did you last manage the money alone?"; hearing loss — "does he answer better when he can see your face?" (2) Because an acute change with fluctuation is the signature of delirium, usually from infection, constipation, dehydration, medication or pain, all treatable; the family has usually been given a diagnosis on the basis of a five-day event and has organised months of decisions around it.

(3) The workload sits with the person who has least authority and the authority with those who do least work; the referral arrives when the worker's capacity fails. (4) Who is responsible; what counts as good care; what the carer is entitled to; what happens if the carer refuses. (5) Because the meeting is about Mama's life rather than her medication, and the order on the page is a statement about what the work is for. (6) The responsible child and the absent ones; the child who is not a child; placement versus refusal; the siblings not speaking — the move for the non-kin carer is to give them standing explicitly, in the meeting, by the family's agreement. (7) Because the conflict is decades old and is not the therapist's business; the right aim is six weeks of concrete cooperation on one practical matter. (8) That the three-month-old "progression" was probably a delirium — found by dating the change and asking what had been done about it.

12.14 Reflective task

Ask one person over seventy the six life review questions, and record the answers if they agree. Then answer three questions of your own: which question produced the most, which produced the least, and what did you have to stop yourself from doing while they spoke? Most students report that they wanted to reassure, to interpret, or to fill a silence, and that the silence was where the material was.

12.15 Further reading

- Erikson, E., Erikson, J. and Kivnick, H. Vital Involvement in Old Age — the empirical follow-up to the final stage.
- Stroebe, M. and Schut, H. "The dual-process model of coping with bereavement" — the source, and the evidence.
- Zarit, S. et al. on caregiver burden — the instrument, and its limits in the room.
- Butler, R. "The life review" — the original paper, still the best description of the task.
- Livingston, G. et al. on dementia care and carer interventions — the best current summary of what works.

Next: Week 13 — Culture, poverty and development: the critique of the norms.

Week 13 — Culture, poverty and development: the critique of the norms

Development is not culture-free. Every norm in the preceding twelve weeks was measured somewhere, on somebody, and the somewhere and the somebody matter. This week supplies the critique, and the critique is not an optional add-on for students with an interest in culture; it is a correction to the instrument they have been learning to use.

The four critiques you must be able to state.

1. **The norms are Western.** Attachment classifications, individuation timetables and "school readiness" were normed on particular populations. A child who sleeps in the parental bed until five is not necessarily enmeshed; in many cultures this is ordinary.

2. **Poverty changes development.** Chronic stress, food insecurity, crowding and insecure housing affect attention, sleep, and the capacity for consistent parenting. Diagnosing "inconsistent parenting" without naming material conditions is a category error.
3. **Extended and distributed caregiving is normal.** In much of the world, children are raised by networks — grandparents, aunts, older siblings, co-wives, foster kin. Attachment theory travels better when it is asked "who does this child go to when she is frightened?" rather than "is she securely attached to her mother?"
4. **The measurement is part of the finding.** A test administered in a second language, by a stranger, in a clinic, to a child who has been told to be quiet, measures the test situation as well as the child. This is not a reason to abandon measurement; it is a reason to interpret it with the conditions attached.

Why this week is placed here and not earlier. Because a critique of the norms is only usable by someone who knows the norms. A student who has not spent twelve weeks learning what typical development looks like cannot evaluate the claim that the norms are parochial; they can only repeat it. By this point in the course the student has the instrument, and this week teaches them what it does not measure.

What the week is not. It is not a claim that development is entirely relative, that milestones are meaningless, or that any practice is defensible because it is cultural. Children die from practices that are cultural, and the therapist who cannot say so has abandoned the child. The position is narrower and harder: the norms describe one pathway in detail, other pathways exist and are not deficient, and the clinical question is always what this child, in this household, with these resources, needs in order to develop.

> **CASE**
> A four-year-old sleeps in her parents' bed in a two-room flat shared with her father's brother. The nursery nurse who referred her has written "over-attached, does not self-soothe". The family is offended and has come only because the school insisted. Assessment finds: the child plays richly, separates without distress, has three adults who respond to her, and has no bedroom. The referral's central "finding" is a description of the housing.

13.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The problem	Why a norm is a measurement plus a population, and what happens when the population is missing	Write the population behind one milestone they know
8–22	The Western norms	Attachment, individuation, readiness — what was measured, on whom, and by whom	Interrogate one classification system
22–36	Poverty as a developmental variable	The five mechanisms, and the evidence for each	Trace one mechanism through a case they have seen

Minutes	Segment	What the lecturer does	What students do
36–48	Distributed caregiving	The network as the unit; the "who does she go to?" question	Map a child's caregiving network from a description
48–60	The measurement problem	Language, stranger, setting, instruction — four contaminants	Design a fair assessment for a named child
60–72	The interpreter	Working with a third person in the room, and the four rules	Practise a three-way conversation
72–82	When to hold the line	The practices a therapist must not relativise, and how to say so	Argue one case both ways and then decide
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. The risk in a week on culture is that it becomes either a sermon on sensitivity or a defence of the discipline. The lecture avoids both by making the class do measurement: give them a milestone chart and ask them to write down the population it was normed on. They cannot, and the inability is the lesson. The instrument they have been using all term is precise and its precision has a location.

The sentence the lecture must land. "Every norm is somebody's average. Your job is to know whose, and then to decide what this child needs."

13.2 The four critiques worked

1. The norms are Western, and more specifically they are from a handful of studies. The Strange Situation was developed in Baltimore in the 1970s on a sample of about a hundred infants; its classifications have been replicated in dozens of cultures, which is a genuine strength, and the distribution of classifications varies in ways that are informative. In several Northern German samples, avoidant classification was more common; in several Japanese samples, resistant. Neither distribution indicated a population of insecure children; it indicated a population of caregiving practices. The classification is a description of a relationship under a particular stressor, not a score of maternal quality.

The same applies to individuation. The timetable in which a child sleeps alone, weans early, and separates for nursery at eighteen months is a description of a particular household economy. A child who sleeps with her mother until five and is carried until three is not delayed; she is participating in a practice with its own developmental logic, and the evidence on later independence does not support the claim that early separation produces it.

2. Poverty is a developmental variable with mechanisms, not a background fact. The mechanisms are five, and each is worth knowing because each is addressable.

- **Chronic stress physiology.** Poverty is associated with elevated cortisol and with alterations in the developing stress response, which affects attention, sleep and emotional regulation. The mechanism is biological and the cause is material.

- **Food insecurity.** Both under-nutrition and the unpredictability of food affect growth, attention and behaviour. A child who has not eaten since the previous evening is not in a condition to learn, and the behaviour that results is routinely read as a disorder.
- **Crowding.** Sleep disruption, no space for homework, no privacy for an adolescent, and no quiet for a child with a hearing loss. Crowding is measurable and is almost never measured.
- **Housing instability.** Moves disrupt schooling, friendships and the continuity of care, and the disruption is frequently read as the child's difficulty with change.
- **Parental bandwidth.** A parent managing money, transport, water, safety and a job has less attention for the ordinary work of parenting, not because they care less but because attention is finite. This is the finding that most changes a clinician's practice, because it converts "inconsistent parenting" from a character judgement into a resource calculation.

3. Distributed caregiving is the norm in most of the world. A child may be raised by a grandmother during the week and a mother at weekends, by an aunt in the city and parents in the village, by an older sister for most of the day. The clinical task is not to assess whether the arrangement is optimal by a Western standard; it is to ask whether the child has at least one adult who is reliably available and responsive, and whether the adults are coordinated.

The question that replaces the Strange Situation in practice: "when she is frightened or hurt, who does she go to?" Followed by: "and does that person know that they are the one?" The second question is the one that finds the problem, which is usually a coordination problem rather than a deficit of care.

4. The measurement is part of the finding. Four contaminants, all of which the therapist can control.

- **Language.** A test in a second language measures proficiency, not ability. Assess in the stronger language, or use a nonverbal instrument, or bring an interpreter — and record which.
- **The stranger.** A child who has been taught not to speak to unfamiliar adults will perform poorly with a clinician and well at home. Observe in the home, or with the caregiver present, or both.
- **The setting.** A clinic room with a stranger and a parent who is anxious is a particular social situation, and the child's behaviour in it is a response to that situation.
- **The instruction.** A child who has been told that children should be seen and not heard may answer "I do not know" to every question, accurately, as a matter of manners.

The correction that follows. Report the conditions with the finding. "Receptive language age equivalent three years six months, assessed in English, second language, with mother present, in clinic" is a substantially different statement from "language delay", and the difference is the whole of this week.

13.2a When the norms are right and the family is wrong

The week's hardest teaching point, and the one that separates a culturally competent clinician from a relativist.

The four categories. A practice may be different and equivalent (co-sleeping, early weaning, grandparent care), different and better (a caregiving network that gives a child four adults instead of two), different and harmful (female genital cutting, corporal punishment to the point of injury, withholding food as discipline, a child kept out of school to work), or not a practice at all but a consequence (crowding, hunger, an untreated infection).

The therapist's obligation in the third category. Name it, once, plainly, and then act according to the law and the safeguarding framework. The order matters: name it to the parents first, in the family's own terms, before any referral. A parent who learns from a social worker what the therapist thought has been humiliated, and the humiliation will be remembered by the whole extended family.

How to name it without a lecture. Three sentences. First, the practice, described factually: "you told me that when he is disobedient he is beaten with a cable." Second, the concern, in developmental terms: "I am worried about his body and about what it teaches him about being hit by people who love him." Third, the position: "I am not able to work with a plan that includes that, and I would like to find something else that works."

What not to do. Do not relativise harm ("in your culture that is normal"), which abandons the child. Do not moralise ("that is child abuse"), which ends the relationship before any change is possible. Do not report without telling the parents, which is sometimes legally required and is always clinically costly and must be explained as it happens.

The position in one sentence. Culture explains a practice; it does not decide whether the practice is good for this child. The therapist's obligation is to hold both halves of that sentence at once, which is what makes the work difficult and what makes it worth doing.

13.2b Three network configurations and what each needs

Distributed caregiving is not one arrangement. Three configurations account for most of the households in these clinics, and each has a different clinical question.

Configuration 1 — The grandmother as primary carer. The mother works, the grandmother raises the child from six months, and the mother resumes the primary role at school age. The child is well cared for and attached, and the transition at school age is the clinical event. The presenting difficulty is usually "he does not listen to me" — addressed to the mother, by a child who has had three years of a different authority. The question is not whether the arrangement was right; it is how the transfer of primary authority is conducted, and it is conducted best when it is explicit and gradual, with the grandmother's status preserved rather than retired.

Configuration 2 — The circulating child. A child who moves between households — school terms with an aunt in the city, holidays in the village, a period with a godmother. This arrangement is common, is often economically rational, and produces a specific clinical picture: a child who is competent, adaptable, and reluctant to attach quickly, and

who is described by adults as "he does not settle". The clinical question is not whether circulation is harmful — the evidence is mixed and depends almost entirely on whether the moves were explained and whether one adult remained constant — but which adult has been constant, and whether the child knows that the constancy is intended to continue.

Configuration 3 — The compound. Several related households in one space, with children moving between them freely and discipline distributed across adults. The child has many caregivers and no privacy, and the developmental benefits are real: language exposure, multiple attachment figures, early social competence. The clinical difficulty is coordination: a child with a hearing loss in a compound is spoken to by eleven people in four languages and is described as "confused", and no single adult has the authority to change the arrangement.

The question for all three. "Who is the person this child would go to, and does that person know?" In configuration 1 the answer is usually the grandmother, and the mother does not know it. In configuration 2 the answer is usually an aunt, and the parents assume it is themselves. In configuration 3 the answer is frequently an older sibling, which nobody has noticed.

13.3 The method: a developmental cultural formulation

Six questions, adapted from the standard cultural formulation for use with a child and a family. Twenty minutes, and the answers go in the record because they change the interpretation of everything else.

Question 1 — Who is in this child's world, and what does each person do? Not the household roster. The functions: who feeds, who puts to bed, who takes to the clinic, who disciplines, who plays, who is trusted with a secret at eleven years old. The answer is the network, and it is frequently larger than the referral suggests.

Question 2 — What is this family's account of the problem, and what does it predict? Families hold theories — spiritual, developmental, characterological, medical, or a mixture — and the theory predicts what they will do. A family that believes the problem is spiritual will pursue prayer; a family that believes it is character will pursue discipline. The therapist who does not ask will recommend something the family cannot use.

Question 3 — What has the family already tried, and what did it cost them? The answer is the family's competence and its budget. A family that has spent four months' income on a treatment that failed is not unmotivated; it is exhausted and poorer.

Question 4 — What are the material conditions? Income, food, housing, water, transport, crowding, the number of people per room, the distance to the clinic. Asked plainly, as part of the history, not as a delicate enquiry. Families answer these questions readily when they are asked as facts rather than as suspicions.

Question 5 — Who has the authority to decide, and who has the authority to be believed? These are frequently different people. A grandmother may decide and a pastor may be believed. The plan must be addressed to both.

Question 6 — What does this family want for this child, in ten years? The question produces the family's own developmental goal, which is often more ambitious than the clinic's and always more motivating. It also frequently resolves an apparent cultural conflict: a family that wants its daughter to be a doctor will accept a good deal of developmental advice about her schooling, provided the advice is connected to that goal.

How to use the six. As the frame for a first session with any family whose background differs from the therapist's — and, since the therapist's own background is always particular, as a frame worth using with every family.

13.3a The interpreter

Many of the assessments in this week's work are conducted through a third person, and the third person changes the data.

Rule 1 — Brief the interpreter before the family arrives. Five minutes: what the assessment is for, what you will ask, and the two things you need — first person interpretation ("she says she cannot sleep", not "she says she cannot sleep and I think she is depressed") and no summarising. An interpreter who has not been briefed will summarise, edit, and add opinions, and all three are invisible in the transcript.

Rule 2 — Speak to the family, not to the interpreter. "How long has she not been sleeping?" is asked of the mother, with the eyes on the mother. The commonest error is the triangle in which the therapist and the interpreter talk to each other about a family sitting in the room.

Rule 3 — Build in double the time. Everything takes twice as long, and the assessment that is scheduled for forty minutes will take eighty. A therapist who runs out of time will cut the questions that are hardest to ask, which are the ones that matter.

Rule 4 — Do not use a family member. Not a child, not a cousin, not a neighbour. The information is distorted by loyalty, the family member learns things they should not, and the roles in the household are permanently altered. Where no interpreter is available, reschedule. This is the one rule in the section that should never be broken.

The fifth rule, which is not a rule but a caution. An interpreter from the same community may be known to the family, and the family may withhold material for that reason — a stigma, a relative, a dispute. Ask, at the start, whether they are comfortable, and offer a telephone interpreter from elsewhere if they are not.

13.3b The sentence that opens the conversation

One practical matter that determines whether the rest of the week's method is usable.

Families are frequently asked, by clinics, to explain their culture — "tell me about your customs" — and the request produces either a performance or silence, because nobody describes their own ordinary life as a custom. The question that works asks about a specific recent event and lets the practice appear inside the answer.

The four openings.

- "Tell me about yesterday. Who was with her from morning until she slept?" — produces the network and the practices in one answer.
- "When she was last ill, who decided what to do?" — produces the hierarchy of authority and the explanatory model.
- "When she does something you do not want her to do, what happens in your house?" — produces the discipline practice without asking for it.
- "What do you want for her that your parents did not have, and what would have to be different in your house for her to have it?" — produces the family's own developmental goal, and it is almost always the most useful sentence in the session, because it is the one that makes the family's own goal available to the plan.

What these four have in common. They ask about events rather than about identity. A family asked to describe its culture must decide what the therapist wants and then supply it; a family asked what happened yesterday simply reports, and the culture arrives in the report, where it can be worked with.

The mistake to avoid. Do not say "I understand your culture" — a claim no outsider can make and every family hears as a claim to authority. Say instead what you noticed: "in your house three people look after her and they all have a view. In mine it was one. Tell me how that works for you, and what you would change about it if you could." That sentence gives the family something to correct, and being corrected is how a therapist learns, which is a slower and more reliable method than being told about a culture by a textbook.

13.4 Three techniques

Technique 1 — The network map of a child. Draw the child in the centre, and around her every adult and older child who does something for her, with the function written next to each name. Then mark the one person she goes to when frightened. Then mark the person each of the others believes she goes to. The discrepancy between the last two markings is the finding, and it is frequently the whole of the case.

Technique 2 — The resource inventory. A list of what the household has, not what it lacks: adults, space, income, a church or mosque community, a neighbour, a skill, a relative in another city, a school with a good teacher. The inventory is written with the family, and it is read back to them. Its effect is not sentimental; it changes what the family believes is possible, which changes what they will attempt.

Technique 3 — The two-column budget. One column: what the plan requires in time and money. The other: what the household has. The technique prevents the therapist from writing plans that require a car, a morning off work, or a fee. It is unglamorous and it is the reason some plans are carried out.

13.4a The assessment kit for a household with nothing

A practical inventory of what a therapist can do when the household has no money, no car, no quiet room and no time.

What costs nothing and is worth doing. A hearing screen with a whispered instruction from behind the child, at a metre, in a quiet moment. A language sample — five minutes of the child talking about anything, recorded on a telephone, transcribed roughly, and counted for mean length of utterance by hand. A drawing. A three-step instruction, given in the corridor. An observation of the child with the person she goes to. An observation of the child with the person she does not go to. None of these requires a kit, a fee, or a second appointment.

What the clinic should supply because the family cannot. A referral letter typed and signed in the session, so that the family leaves with it rather than waiting for a post that does not arrive. A telephone number that is answered. A note for the school. A copy of the assessment in a form the family can keep.

What not to ask for. Do not ask a family to return on a day they cannot manage, to bring a document that does not exist, or to obtain a report from a school that is not answering. These requests are made in good faith and they are the mechanism by which families are lost: each one is a small obstacle, and the cumulative effect on a household operating at capacity is a decision not to come back.

The design principle. Every element of the assessment should be deliverable in one visit, by one person, with no equipment, and end with the family holding a piece of paper. That constraint produces a shorter assessment than the textbook describes and a more useful one, because it is completed.

13.4b The four-column plan

The plan for a household under material constraint is written in four columns, and the fourth is the one that keeps it alive.

Column 1 — What we are trying to change. One sentence. Not a list.

Column 2 — What it requires. In time, money, transport and space. Written honestly, including the invisible costs: a morning off work, a fare, a person to mind the other children.

Column 3 — What the household has. From the resource inventory. The column is frequently fuller than the family expected, which is part of its purpose.

Column 4 — What we drop if column 2 exceeds column 3. The reduction, specified in advance. Most plans fail because they were designed at full capacity and nobody decided what to cut when capacity fell. A plan with a stated reduction survives; a plan without one is abandoned whole.

The example. Column 1: "we want him to be understood at school." Column 2: a hearing test, an audiology appointment across the city, a school letter, and ten minutes a day of reading aloud. Column 3: the mother's mornings are free on Tuesdays; there is a bus; the school has a teacher who has been helpful. Column 4: if the audiology appointment cannot be reached, the school letter and the ten minutes continue and the appointment is rebooked; if the ten minutes cannot be managed, the school letter continues alone.

What the column prevents. The all-or-nothing plan, which is the characteristic failure of clinicians working with families in difficulty, and which is experienced by the family as another expectation they could not meet.

13.5 The first session when the family and the therapist disagree

Phase 1 — Ask for the family's account before offering any. The six questions, and particularly question two. A therapist who offers a developmental account in the first ten minutes has produced a competition between two theories, and the family will defend theirs.

Phase 2 — Find the point of agreement. It is almost always the goal: the family wants the child to do well at school, to be safe, to be respected. State it: "we want the same thing for her."

Phase 3 — Offer the second language, not a replacement. "Your grandmother is right that a child needs to be taught respect. I would add something from my side, which is that a child who is shouted at every morning finds it harder to concentrate at school, and concentration is what she needs for the exam." The move is additive: the family keeps its framework and gains a mechanism.

Phase 4 — Say clearly where you cannot go. If a practice is harmful, name it once, as in 13.2a, and hold the line. Ambiguity here is worse than refusal, because the family will hear permission.

Phase 5 — Agree one thing and a review. As in Week 11. One action, dated, with the exception named.

What to do when the family does not return. Assume the plan was unusable rather than that the family was unmotivated, and ask. A telephone call that says "I have been thinking about what we agreed and I wonder whether it was possible in your house" recovers a substantial minority of families who have dropped out, and no service does it.

13.5a When the therapist's own culture is the variable

The week's critique applies to the clinician, and the application is uncomfortable.

What the therapist brings. A household of origin with its own practices — sleeping arrangements, discipline, the meaning of respect, the proper distance between a mother and a grown son, the age at which a child is left alone. A training in a discipline that was normed somewhere. And an institution's assumptions, which are frequently the assumptions of the profession's country of origin rather than of the clinic's neighbourhood.

The three ways this shows up.

As an unexamined standard. The therapist who is irritated by a family and cannot say why is usually meeting a practice that violates a rule she has never stated. The correction is to state the rule: "I notice I am uncomfortable that the eight-year-old is still sleeping in the room with his parents. That is my household's rule, not a finding. What is the reason in yours?"

As a class position. A therapist from a professional household assessing a family in poverty may misread scarcity as disorganisation, or may over-correct and fail to name a real problem. Both errors come from the same place and both are corrected by asking about the material facts rather than inferring them.

As an alliance with the institution. The therapist is employed by a clinic, often by a school, sometimes by a court. Her assumptions drift toward her employer's, and the drift is invisible from inside. The correction is to ask, once per case, "whose interests does my formulation serve?" and to write the answer in the record.

The discipline that holds all three. State your own rule before you interpret the family's. The sentence "I was brought up to think X, and you are describing Y — tell me about Y" is not a confession; it is the technique. It removes the therapist's rule from the position of an unstated standard and puts it on the table where the family can examine it, which is the only position from which a family can disagree with a professional without feeling judged.

13.6 Case: the child with no bedroom

Referral. Morenike, four years and three months, referred by the nursery attached to a private school. The written note: "over-attached to mother, does not self-soothe, still in parents' bed, will not sleep alone. Family has been advised to implement a sleep programme."

The family. Yetunde, 29, a hairdresser who works from the flat. Bayo, 32, a dispatch rider. Morenike, 4. Bayo's younger brother, 24, a student, who sleeps in the sitting room. Total: four people in a flat of two rooms and a sitting room, in a compound of six flats. Languages: Yoruba at home, English at nursery.

Why they came. "The school said they will not promote her to the next class if she is not sleeping alone." Yetunde's tone, in the first minute: "they have told me I am spoiling her. My mother says the same. I want to hear it from a doctor."

What the assessment finds. Morenike plays richly, with a narrative and with other children; she separates from her mother at nursery without distress and greets her warmly on reunion; she has a vocabulary above age in Yoruba and age-appropriate in English; her hearing is normal; her sleep is unremarkable apart from its location — she goes to sleep at nine and wakes at six. When frightened, she goes to her mother. Her mother knows this. Her father believes she goes to her grandmother, who lives two compounds away.

The five findings.

1. There is no evidence of a sleep disorder and there is no spare room. The family occupies two rooms: one for Yetunde and Bayo, one for Morenike and her cousin when the cousin stays. The uncle sleeps in the sitting room. "Sleeping alone" would mean sleeping in a room with a twenty-four-year-old man, which the parents correctly regard as unacceptable. The clinic's recommendation was not a developmental intervention; it was a recommendation to rearrange a household that has no space, made by someone who did not ask.

2. The referral is a description of the housing written in the language of attachment. "Does not self-soothe" describes a child who has never been alone in a room. "Over-attached" describes a child in a two-room flat. Neither phrase is false and both are diagnoses of a circumstance.

3. The child's attachment behaviour is unremarkable. She separates, she explores, she returns. The Strange Situation, if it had been administered, would have had to be administered in a room she had never seen with a stranger she had never met, and her behaviour in that room would have described the room.

4. There is a real question, and it is about the school's requirement. The school's promotion criterion is being used as a developmental standard, and it is not one. The therapist's task is not to adjudicate between the family and the school on a matter of principle; it is to give the family a position and a sentence they can take to the school.

5. The family's own goal is specific and achievable. They want her to sleep in her own bed by the time she starts Primary 1, in fourteen months, so that the uncle can move out when he finishes his studies and the room can be rearranged. They have a plan and it is better than the clinic's.

The formulation. A well-developing four-year-old in a household with four people, two rooms and no spare bed, referred by an institution that has mistaken a housing constraint for a developmental deficit, and whose mother has been told by three authorities that she is spoiling her child. The intervention is three sentences to a school, one piece of information to a mother, and a plan that begins in fourteen months.

The intervention, in five parts.

Part 1 — Say the finding in one sentence, and say it early. "There is nothing wrong with your daughter's development, and there is nowhere in your flat for her to sleep alone. Those are two different things and the school has joined them together."

Part 2 — Give the mother the developmental fact she can use. "A four-year-old who sleeps in her parents' room in a two-room flat is doing what children in most of the world do. What matters for her development is that she is responded to when she calls, and she is."

Part 3 — The letter to the school, written in the session. Three paragraphs. It states that the child has been assessed, that no developmental concern was found, that the family's sleeping arrangement reflects their housing and not their parenting, that the assessment found no basis for a sleep programme, and that the clinic will review the arrangement at a stated date. It is signed by the therapist and copied to the parents. The therapist reads it aloud before anyone leaves.

Part 4 — A plan for fourteen months, in the family's own shape. A bed for Morenike, bought second-hand, in the room she shares; a routine at nine that is hers; and a target — by the time the uncle finishes, she sleeps in her own bed in the room, and the room is rearranged. Written on a page, with the date.

Part 5 — One sentence for the grandmother. "She is not spoiled; she is four, and we have no room." The sentence is rehearsed, because the grandmother's opinion is the one that reaches the mother every day and the mother needs something to say that is not a defence of herself.

13.6a Five months later

The letter to the school was effective in a way the therapist had not anticipated: the school removed the promotion condition and asked the clinic to speak to the nursery staff, which the therapist did, for forty minutes, on the difference between a developmental assessment and a housing description. Nine children in that class stopped being on a sleep programme.

Morenike has a bed. It is in the same room as her mother's, at right angles, and she sleeps in it most nights. The target date is still fourteen months away and the plan is ahead of schedule, which the family reports with pride.

Yetunde has stopped apologising in the first minute of a conversation, which the therapist regards as the case's most significant outcome and which is not measurable by any instrument in this book.

The honest limitation. The uncle still sleeps in the sitting room and the flat is still two rooms. The therapist has not changed the material condition and does not claim to have done so. What she has changed is that a family is no longer being told that its housing is a parenting failure, and that an institution has been corrected, which affects every family that institution sees afterwards.

13.7 Transcript: the letter, read aloud

Nine turns from the third session. The therapist has drafted the letter in front of the parents and is reading it back to them before it is sent.

> **T1 Therapist:** (reading) "To the Head of Nursery. I am writing about Morenike, whom I assessed on the fourteenth. I found no developmental concern. Her play, her language, her separation and her relationships are all appropriate for her age." (pause) Is that right? — The check after the first paragraph. The letter is a clinical document and the family is the authority on its accuracy. **T2 Yetunde:** It is right. **T3 Therapist:** (reading) "The family's sleeping arrangement reflects the size of their home. It is not a sign of developmental difficulty, and I have not recommended a sleep programme." (pause) Yetunde, when you read that, what will you feel? — The question is asked before the letter is sent, because a letter the parent cannot bear to read is a letter that will not be used. **T4 Yetunde:** (quietly) That somebody said it. **T5 Therapist:** (reading) "I would be glad to speak to the nursery staff about the difference between a developmental assessment and a description of a family's housing." (pause) That last sentence is a criticism of the school. Do you want it in? — The choice is offered deliberately. A letter that criticises an institution on a family's behalf, without the family's agreement, converts the therapist into an advocate and the family into a party to a dispute they did not choose. **T6 Bayo:** (for the first time in the session) Leave it in. **T7 Therapist:** Then I will send it today and I will copy you both. (pause) And one more thing. (to Yetunde) Your mother will hear about this letter from the school, not from you. What will you say to her? — The household rehearsal from Week 11, applied. The plan will be tested at home before it is tested at the school. **T8 Yetunde:** (after a pause) That the doctor said she is not spoiled. That we have no room. **T9 Therapist:** Say it again, so that you have said it once before you say it to her.

13.8 Laboratory: assessing across a difference

Aim. To complete a developmental assessment with a family whose language, housing and explanatory model differ from the therapist's, and to report the finding with its conditions attached.

Procedure. Groups of four: therapist, mother, interpreter, observer. The trainer supplies a brief in which the family speaks mainly Yoruba, has no word for "self-soothe", believes the child's night waking is caused by a neighbour's envy, and has four people in two rooms. The therapist may use the interpreter and must produce a one-paragraph written finding in ten minutes at the end.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The six questions	Two or fewer asked	Four asked	All six, in order, with the answers in the family's words
The interpreter	Treated as a participant; spoke to them not the family	Used correctly but not briefed	Briefed, addressed the family, first-person, no summarising
The material facts	Not asked	Asked, and not used	Asked, used, and reflected in the plan
The explanatory model	Ignored, or corrected	Acknowledged	Acknowledged, incorporated, and one mechanism added alongside

Criterion	0–1	2–3	4–5
The written finding	"Language delay, insecure attachment"	A description without conditions	A finding with the language, setting and conditions stated

The distinction that carries most of the marks. The top band requires the written finding to state the conditions of the measurement. "Receptive language age equivalent three years, assessed in English as a second language, with the mother present, in a clinic room" is the standard; "language delayed" is a failure of the exercise however accurate the number.

13.9 Six errors

Error 1 — Treating a housing description as a developmental finding. The week's central error. "Does not self-soothe" and "no bedroom" are the same observation written by two different professions.

Error 2 — Relativism as kindness. "In your culture that is normal" is said to avoid a difficult conversation and it abandons the child. Culture explains a practice; it does not settle whether the practice is good for this child.

Error 3 — Importing the norms without their population. Every instrument in the course was normed somewhere. Using it elsewhere is legitimate; using it elsewhere as though it were normed there is not.

Error 4 — Ignoring material conditions in the formulation. A plan that requires a car, a morning off, a fee or a spare room will fail, and the family will be recorded as unmotivated. The two-column budget exists to prevent this.

Error 5 — Using a family member as interpreter. It distorts the information, alters the household's roles, and cannot be undone. Reschedule instead.

Error 6 — Fighting the institution in front of the family. The letter in the case is a clinical instrument; a therapist who becomes the family's advocate against a school has taken a side in a dispute she will not be present to finish, and the family will carry the consequences.

13.10 Culture, faith and the limits of the critique

The four critiques in this week do not license a fifth, which would be: nothing can be known. The position this course takes is stated here so that students can hold it under pressure.

What is universal enough to rely on. Children need at least one reliably responsive adult; they need food, sleep and safety; they need language input from people who speak to them; they need to be protected from violence; and they develop through play with other children. These are not Western values; they are findings that hold across the ethnographic record, and a therapist who cannot assert them cannot protect a child.

What varies and must not be pathologised. Sleeping arrangements, weaning, who provides care, the age of school entry, the amount of autonomy given to a child, the

manner of discipline within limits, the expression of emotion, the meaning of a disability, the conduct of mourning, and the relationship between the family and institutions.

The test that distinguishes them. Ask of any practice: does it protect or damage the child's development — her body, her language, her capacity to relate, her access to learning? If it damages, it is not protected by culture. If it does not, the therapist has no business treating it as a problem, whatever her own household did.

Faith and the explanatory model. A family's spiritual account of a child's difficulty is not a competitor to the assessment and does not need to be displaced. It needs to be joined, and one mechanism added alongside. The pastor who is told that the clinic is not an enemy becomes an ally who can say, from the pulpit, that a hearing test is not a lack of faith — and that sentence reaches more families than the clinic will see in a year.

> **DEBATE**
> "A developmental assessment should not be conducted across a language barrier at all." Argue for: the instrument measures the language, the conditions invalidate the norm, and a wrong number is worse than no number because it becomes a label. Argue against: no assessment means no service, waiting lists are a form of harm, and a properly-reported assessment with its conditions stated is more useful than nothing. Then state what you would actually do for a five-year-old who speaks only Hausa in a clinic where nobody does, and what you would write in the record.

13.11 Evidence note

What the research supports. The cross-cultural replication of attachment classifications is substantial: the categories appear in every studied population, and the distribution varies with caregiving practice, which supports the theory's universality and undermines any single distribution being treated as a standard. The evidence on poverty as a developmental variable is among the strongest in the field, with the mechanisms — stress physiology, nutrition, crowding, instability — demonstrated across cohorts. The association between hearing loss and apparent cognitive and social difficulty is robust and correction improves outcomes.

What is weaker. Norms for most developmental instruments are drawn from samples that are unrepresentative of the populations in which the instruments are used, and the published corrections are sparse. The evidence on culturally adapted parenting interventions is growing and remains thinner than the evidence for the unadapted versions, which is a problem of research funding rather than of principle.

What is not supported. That early independence training produces later independence. That co-sleeping produces enmeshment. That a child raised by a network is at risk by virtue of the network. And that cultural sensitivity requires suspending judgement — the evidence on child maltreatment across cultures is clear enough that a therapist who cannot name harm is not sensitive but negligent.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same sleeping arrangement is

unremarkable in one household and a safeguarding concern in another, and what distinguishes them is not the arrangement but the responsiveness of the adults in it.

13.12 Key terms

Norm — a distribution of measurements in a population; a description, not a standard.

Explanatory model — the family's own account of what is wrong and why; it predicts what they will do.

Distributed caregiving — care provided by a network rather than a pair; the norm in most of the world.

Material conditions — income, food, housing, crowding, water, transport; developmental variables, not background.

Bandwidth — the finite attention a caregiver has for the work of parenting, reduced by scarcity.

Cultural formulation — a structured enquiry into the family's world, account, resources, authorities and goals.

Conditions of measurement — the language, setting, relationship and instruction under which a finding was obtained, which must be reported with it.

Relativism — the position that no practice can be judged from outside its culture; incompatible with child protection.

Network map — a diagram of who does what for a child and who the child goes to when frightened.

13.13 Self-test

1. State the four critiques, and give one piece of evidence for each.
2. What are the five mechanisms by which poverty affects development, and which one most changes clinical practice?
3. Give the question that replaces the Strange Situation in a household with distributed caregiving, and the follow-up question that finds the problem.
4. Name the four conditions of measurement and explain how each contaminates a finding.
5. When is a practice harmful rather than different, and how do you say so in three sentences?
6. Give the four rules for working with an interpreter, and the one that must never be broken.
7. Why does the therapist ask the family to read the letter before it is sent?
8. In the Morenike case, what was the referral actually describing?

Answers. (1) Western norms — the Strange Situation's distribution varies with caregiving practice across cultures; poverty — the stress-physiology and nutrition evidence; distributed caregiving — the ethnographic record and the network as the normal unit;

measurement — the language and setting effects on test performance. (2) Chronic stress physiology, food insecurity, crowding, housing instability, and parental bandwidth — the last changes practice most, because it converts "inconsistent parenting" into a resource calculation. (3) "When she is frightened, who does she go to?" and "does that person know they are the one?" (4) Language, stranger, setting, instruction — each measures the situation as well as the child, so the finding must be reported with them attached. (5) When it damages the child's body, language, capacity to relate or access to learning; name the practice factually, name the concern developmentally, and state what you are not able to work with. (6) Brief beforehand; speak to the family; double the time; never use a family member — the last must never be broken. (7) Because a letter the parent cannot bear to read will not be used, and because the family is the authority on its own accuracy. (8) The family's housing.

13.14 Reflective task

Write down three things your own household did with children that a textbook would call a problem, and three that it did that the textbook would call correct. Then answer: which of the six did you assume was normal until this week? Most students find at least one in each column, and the discovery that the textbook's standards are also somebody's household practice is the week's actual learning.

13.15 Further reading

- Henrich, J., Heine, S. and Norenzayan, A. "The weirdest people in the world?" — the sampling problem, in general.
- Keller, H. Cultures of Infancy — the comparative evidence on caregiving practice.
- McLoyd, V. "Socioeconomic disadvantage and child development" — the poverty mechanisms.
- Kirmayer, L. on cultural formulation — the structured interview this week adapts.
- Serpell, R. The Significance of Schooling — African evidence on what school readiness actually means.

Next: Week 14 — When development goes wrong: the differential habit.

Week 14 — When development goes wrong: the differential habit

The four categories. For any developmental concern, run the check before you form a hypothesis. Every presentation in this course is one of four things, and the four have different prognoses and different referrals.

Category	What it means	Example	What it usually requires
Variation	Within the normal range, differently timed or differently shaped	Late talking with good comprehension at 22 months	Information, review, no referral

Category	What it means	Example	What it usually requires
Delay	Behind the expected sequence, but progressing	Two-word combinations at three years	Assessment and early intervention
Regression	Loss of a skill that was present	Words at two, silence at two and a half	Urgent medical assessment
Symptom	A behaviour that expresses something else	Night waking after a house move	The thing it expresses

The five systemic questions. Behind every presentation there is a family, and the family has five facts that predict what will happen next.

1. **When did it start, and what else changed then?**
2. **Who noticed first, and who disagrees?**
3. **What does the family do when it happens, and what happens next?**
4. **What would improve if it stopped — and what would get worse?**
5. **What has already been tried, and what did that teach us?**

The differential table. The table below is the instrument, not a reference. It is used in the first session, from memory, for any presentation in the course.

Presentation	Developmental differential	Systemic differential
Aggression at 5	Language delay; impulsivity; trauma	Parental conflict; inconsistent limits; a new baby
Withdrawal at 12	Depression; bullying; neurodivergence	Family secrecy; a parent's illness; migration
Decline at 70	Delirium; dementia; depression; hearing loss	Bereavement; relocation; loss of role
Non-attendance at 9	Anxiety; learning difficulty; bullying	A function at home; an unresolved parental conflict
Clinging at 2	Temperament; separation; illness	A caregiver's own anxiety; a recent change of carer

Why a "habit" and not a "skill". Because the differential is not something a clinician does once; it is something they do automatically, before the formulation forms. The difficulty is that formulations form in about four seconds, from the referral letter, before the family has sat down. This week's purpose is to install a pause between the referral and the hypothesis, and a pause is installed by repetition rather than by understanding.

> **CASE**
> A boy of two and a half who had forty words at twenty months and has none now. His mother is certain. His father says he is lazy. The grandmother says he will talk when he is ready, as his uncle did. The nursery says he is fine. Regression is the one category in which the family's disagreement is not a family problem but a clinical emergency, and the therapist's first task is to establish the facts before anyone's theory is discussed.

14.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	Student activity
0–8	The four-second formulation	How a hypothesis forms from a referral letter, and why that is the problem	Read a referral and write their hypothesis in one sentence
8–22	The four categories	Variation, delay, regression, symptom — with examples from earlier weeks	Sort twelve presentations into the four
22–34	Regression	Why it is the one urgent category, and what it demands	Identify the three facts that establish it
34–48	The five systemic questions	The questions that predict what the family will do	Run the five on a case, in pairs
48–62	The differential table	Worked from memory, from four presentations	Reproduce the table for two presentations
62–74	The red flags	The specific findings that require a same-week referral	Match eight red flags to their urgency
74–84	Referral and its aftermath	How to refer well, and what to do while the family waits	Write a referral letter
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. The lecture opens by having every student write their hypothesis from a referral letter in one sentence, before any discussion. The sentences are read aloud, and they are almost always characterological ("anxious mother", "difficult child", "attention-seeking"). The class then works the same case properly and discovers how little the letter supported. The exercise is uncomfortable and it is remembered, which is what a habit requires.

The sentence the lecture must land. "The referral letter is a description of somebody's worst day, and your first job is to find out which of four things you are looking at."

14.2 The four categories worked

Variation. The child is developing along a recognisable path at a different rate or in a different shape. The wide range of normal at every milestone is the single most under-used fact in clinical practice: walking anywhere from nine to eighteen months, first words from nine to eighteen months, two-word combinations from eighteen to thirty months, bladder control from two to four years. A child at the far end of any of those ranges, with good comprehension, good play, good relating and no regression, is a child about whom the correct action is review.

The clinical discipline of variation is to say so plainly. Parents are frequently more reassured by a number than by a sentence — "the normal range for this is nine to eighteen months and he is fifteen" — and giving the range is not false reassurance; it is the finding.

Delay. The child is behind the expected sequence and progressing slowly. The distinguishing feature is not the gap but the trajectory: a child who is six months behind and gaining a month a month is closing; a child who is six months behind and gaining a month every three is falling further behind. The trajectory requires two measurements,

which means the assessment is not complete in one visit, which is why the review date is part of the finding rather than a courtesy.

Regression. The child has lost a skill. It is the only category that is always urgent, for three reasons: some causes are treatable and time-critical (a space-occupying lesion, a metabolic disorder, epileptic encephalopathy, a hearing loss from infection, severe iron deficiency); some are progressive; and the family's account of a regression is frequently disbelieved by the first professional they see, which costs months. The therapist's obligation is narrow and absolute: establish whether it is real, and if it is, refer the same week.

Symptom. The behaviour is an expression of something else — a change, a loss, a conflict, a fear, an illness. This is the category the rest of the course has been teaching, and it is the one most often over-applied: a therapist trained in family systems will find a systemic explanation for a regression that needs an MRI. The four-category check exists precisely to prevent the reverse error, and the therapist's discipline is to run it first, even when the systemic explanation is obvious.

The fifth possibility that is not a category. Nothing. A substantial proportion of referrals to child services describe a child who is developing normally in a family that is under strain, and the correct formulation is "no developmental concern; a family that needs help". That formulation is not a failure to diagnose; it is a diagnosis, and it should be written down as clearly as any other.

14.2a The five systemic questions, worked

Each question has a purpose, a common wrong answer, and a follow-up that defeats it.

Question 1 — When did it start, and what else changed then? The purpose is to date the onset relative to events, which is the entire method of Week 1. The common wrong answer is a duration ("about a year"), which is unusable. The follow-up: "what was happening in the house that month — a move, a birth, a death, a new job, someone leaving?" Families can usually date an event even when they cannot date a behaviour, and the event dates the behaviour.

Question 2 — Who noticed first, and who disagrees? The purpose is to find the household's fault line. The disagreement is data: it tells you who has the child during the day, whose authority is contested, and which of the adults will not support the plan. The follow-up: "what does he say when you tell him?" The answer frequently reveals a second, older disagreement.

Question 3 — What does the family do when it happens, and what happens next? The purpose is the behavioural sequence, and it is the question that produces the intervention. The follow-up matters more than the question: keep asking "and then what?" until the sequence closes. A typical sequence: he cries, she shouts, he cries harder, she gives him the phone, he stops. The phone is the intervention's target, and nobody in the family has described the sequence as a sequence.

Question 4 — What would improve if it stopped, and what would get worse? The systemic question, and the one students forget. A symptom does something in a

household, and its removal changes the household. The follow-up: "who would notice first?" This question identifies the person whose position depends on the problem, and that person is frequently the one who will not cooperate.

Question 5 — What has already been tried, and what did that teach us? The purpose is the family's competence and its budget. The follow-up: "what happened when you stopped?" A family that tried a plan and abandoned it has usually learned something about what is feasible, and the therapist who does not ask will recommend the same plan again.

Why the five are asked in this order. Question 1 establishes the timeline, which the other four depend on. Question 2 finds the fault line, which tells the therapist whom to address. Question 3 produces the behavioural sequence. Question 4 is asked only after the sequence is known, because it is a question about function and function cannot be guessed. Question 5 produces the plan's constraints, and asking it earlier tends to convert the interview into a negotiation.

14.2b Red flags: the findings that require a same-week referral

The list below is not exhaustive and is not a licence to practise medicine. It is the minimum that a family therapist must recognise in order to refer rather than to treat.

Regression of any kind. Loss of words, loss of walking, loss of bladder control after a year of dryness, loss of social smiling, loss of a skill that was established. Urgent.

Loss of vision or hearing, or a change in either. A child who stops turning to sound, or who begins to hold objects very close. Urgent.

A change in consciousness or awareness. Episodes of "switching off", staring that cannot be interrupted, a child who does not respond to pain. Urgent.

Headache that wakes the child, or vomiting on waking, or a new squint, or a change in gait. Urgent.

Growth failure. Weight loss, or a fall across two centile lines, or a child who is not gaining. Urgent.

Any symptom that is worse in the morning and better later, which is the pattern of raised intracranial pressure, and any symptom that is steadily progressive.

Seizures, or suspected seizures, including episodes the family describes as "he goes quiet and his eyes roll".

Self-harm, suicidal ideation, or a plan in any child old enough to hold one, at any age. Same day.

Sexual knowledge or behaviour beyond the child's developmental stage, or any disclosure of abuse. Same day, safeguarding framework.

A parent's account that does not fit the injury. The safeguarding question, asked every time, without exception, and without requiring certainty.

The discipline. The therapist does not need to know what the condition is. She needs to recognise the category, say so plainly to the family, and make the referral in writing before the family leaves the building. "I do not know what is causing this and it needs to be looked at this week" is a complete and honest clinical statement, and it is far better than a formulation that keeps the family in the therapy room for three months.

14.2c The child who is referred twice in a year

A configuration that tests the four-category habit more than any single presentation: the same child, returned to the clinic, with the same complaint, by a different professional.

What the second referral usually means. One of four things. The first assessment found nothing and nobody said so clearly. The first intervention was not implemented, for reasons that were material rather than motivational. The complaint has changed in a way nobody recorded. Or the first formulation was wrong.

The four questions that distinguish them, asked in this order.

1. "What has changed since the last time he was here?" If nothing has changed in the child and something has changed in the setting — a new school, a new teacher, a new baby — the second referral is about the setting.
2. "What were you told to do, and what happened when you did it?" If the family did the action and it failed, the formulation is probably wrong. If the family did not do it, the reason is the finding.
3. "Who sent you this time, and what did they say?" The referral's source is information: a school that refers twice is telling you about the school's capacity, not only the child's difficulty.
4. "What would have to change for you not to need to be here?" The family's answer is the outcome measure, and it is frequently something the first assessment never addressed.

The trap. The second referral invites the therapist to work harder on the same formulation. The discipline is to work the four-category check again from the beginning, as though the child were new, because one time in five they are.

The record. A second referral should produce a note that says explicitly which of the four explanations applies and what the evidence is. A file that accumulates four assessments and no conclusion is a file that will produce a fifth, and the fifth will be the one in which somebody finally notices that the question itself was the wrong one.

14.5a The assessment the family can afford

A short section with a large practical consequence, placed here because this week generates more referrals than any other.

The arithmetic. A specialist appointment may cost a fare, a day's income, a night's accommodation and the care of other children. For a household with three children and a daily wage, the true cost of a referral is frequently more than a month's discretionary income, and the therapist who does not compute this will make referrals that are not attended and will conclude that the family is not motivated.

What to ask, every time, before the referral is written. "What would it take for you to get there?" Then listen for the four costs: money, time, distance, and the care of the other children, which is the cost most often omitted from a clinician's arithmetic and the one most often decisive. Then solve the ones that can be solved. A referral to a Thursday morning clinic for a mother who works Thursday mornings is not a referral; it is a suggestion.

The three practical moves. Ask the receiving service whether there is a cheaper or closer site, which there frequently is and which nobody has asked about. Write the letter in a way that lets the family be seen once rather than three times, by putting the trajectory and the red flags in the first paragraph. And give the family a single named contact at the clinic who will take a telephone call, which reduces the number of journeys and is the therapist's own job rather than a system's.

What cannot be solved, and must be said. Sometimes the appointment is unaffordable and the service is distant, and the honest response is to say so, to write to the service, and to be explicit with the family about what the clinic will do in the interval. A family told the truth about a difficulty can plan around it; a family told that help is coming cannot.

14.3 The method: a differential assessment in one session

Step 1 — Establish the facts of the trajectory, not the level. Two questions: "what could he do six months ago that he cannot do now?" and "what can he do now that he could not do six months ago?" Every assessment that opens with these two questions is better than one that opens with a milestone checklist, because it produces a direction.

Step 2 — Place the presentation in one of the four categories, provisionally, and say so. "On what you have told me this looks like a delay rather than a regression, and I will tell you if that changes." Families find provisional formulations more usable than certainty, because a provisional formulation leaves room for their knowledge.

Step 3 — Run the red-flag list. Thirty seconds. If any is positive, stop the assessment and refer.

Step 4 — Run the five systemic questions. The timeline, the fault line, the sequence, the function, the attempts.

Step 5 — Complete the four-category check with the evidence you have. If the evidence is insufficient, say which category is most likely and what would settle it — an audiology appointment, a developmental assessment, a second measurement in three months.

Step 6 — Write the differential in the record, in order of probability, with the reasoning. Not a diagnosis. A differential with reasons is what a clinician needs six months later, and it is what protects a family from being reassessed from scratch by every professional they meet.

Step 7 — Decide the referral, and make it in the room. Typed, signed, copied, with a telephone number and an expected waiting time. And say what happens if the waiting list is long: what the family will do in the meantime, and who will chase it.

Step 8 — Book the review before the family leaves. Even where a referral has been made. The review is the therapist's own second measurement, and it is the thing that catches the case in which the first formulation was wrong.

14.3a Referring well, and what to do while the family waits

The letter. Six elements: the reason for referral in one sentence; the trajectory, with dates; what the four-category check found; the red flags that are absent and the ones that are present; what has been tried; and what the referrer wants. One page. Addressed to a named service, with a named person if possible, and copied to the family.

The chase. A telephone call at two weeks and at six weeks. In most systems a referral that is not chased is a referral that is lost, and the loss is invisible to everyone except the family, who conclude that nothing can be done.

The waiting period. The family is waiting for a service that may take months. Three things can be done in the meantime and each is worth doing: protect sleep; give the child one domain of competence; and give the family one explanatory sentence they can use with relatives, which reduces the pressure that usually falls on the mother.

The wrong thing to do while waiting. Do not begin a treatment for the condition you suspect before it has been assessed, because the treatment will muddy the assessment. Do not tell the family that the referral is a formality. And do not stop seeing them: the referral is to a service, not away from the therapist.

14.3b Reading somebody else's assessment

Most children who arrive at a family therapist have been assessed before, and the previous report is frequently the most misleading document in the file.

What previous reports usually contain, and what each thing means. A score, without the conditions under which it was obtained. A diagnosis, frequently made at the end of a single appointment. A recommendation, frequently written for a service that no longer exists. And an absence — the things nobody asked, which are invisible because they are not written.

The four questions to ask of any previous assessment.

1. **Who did it, in what language, with whom present?** Week 13's conditions of measurement, applied to a document rather than to a session.
2. **What was the child doing at the time?** Children are assessed during illness, after a move, in the week a sibling is born. A score obtained in a bad month is a score of the month.
3. **What has changed since?** A report is a photograph. The family's own account of the last year is the film, and it is usually more informative than the report.
4. **What did they not ask?** The hearing. The sleep. The food. The school. The grandmother. These are the five omissions in most of the reports this book's students will read.

The rule for using a report. Quote it with its date and its conditions, and never inherit its conclusion without its evidence. "A developmental assessment in June last year found a language delay; I do not have the scores or the conditions, and I will ask for them" is a professional sentence. "He has a language delay" said on the basis of another clinician's paragraph is not.

When to ask for the original. Whenever a decision depends on it. A request for the full report, with the scores and the conditions, is a legitimate professional act and takes one telephone call.

The family's experience of repeated assessment. A child who has been assessed four times has usually been assessed well at least once, and the family has usually been told four different things. The therapist's question at the start is not "what did they find?" but "what were you told, and what did you think of it?" The second half is the part nobody asks, and it is where the family's real formulation lives, usually in a sentence the first clinician did not hear because the appointment had already run over its time.

14.4 Three techniques

Technique 1 — The four-category card. A card, carried, with the four categories on one side and the red-flag list on the other. Its purpose is not reference; it is to slow the four seconds between the referral letter and the hypothesis. Clinicians who carry it report that they use it for the first year and then no longer need it, which is the definition of a habit.

Technique 2 — The onset line. A single horizontal line from birth to today, with the onset of the concern marked, and every event the family can date marked above it. The line does two things: it dates the problem, and it makes visible the coincidence that the family has not noticed. It is the Week 1 timeline applied to a symptom rather than a life, and it takes four minutes.

Technique 3 — The two-measurement rule. Nothing that could be a delay is concluded from one measurement. The first assessment produces a level and a hypothesis; the review produces a trajectory. The rule is stated to the family as part of the finding — "I am going to measure this again in three months, and what I am looking for is not a different number but a different direction" — which converts the review from a courtesy into the assessment.

14.5 The first session with "something is wrong"

Phase 1 — Take the account without a hypothesis. Fifteen minutes in which the therapist asks only factual questions and writes down the family's words. The discipline is to say nothing interpretive in this phase, which is harder than it sounds when the account is vivid.

Phase 2 — The two trajectory questions. What could he do six months ago that he cannot now; what can he do now that he could not before.

Phase 3 — The red-flag list, quickly and out loud. "I am going to ask you some quick questions about his health. Most of them will be no." Families accept a list; they become frightened by a vague enquiry.

Phase 4 — The four-category placement, stated provisionally. "This looks like X to me at the moment, and here is what would change my mind."

Phase 5 — The referral, made in the room. Typed, signed, copied, explained.

Phase 6 — One thing for the wait, and the review date. Both written.

Why the hypothesis is withheld in phase 1. Because a family that hears a hypothesis in the first five minutes organises everything afterwards to support or refute it, and the assessment then measures the family's theory rather than the child.

14.6 Case: the boy who stopped talking

Referral. Mrs Okafor telephones about her son, Chuka, two years and seven months. "He has stopped talking. He used to say words. Now he says nothing. My husband says he is lazy. My mother says his uncle was the same and he talks now. The nursery says he is fine."

The household. Mrs Okafor, 31, a teacher. Mr Okafor, 36, a civil servant, posted to another state and home at weekends. Chuka, 2;7. A sister, 8 months. The paternal grandmother, 63, who lives with them and who raised Mr Okafor's younger brother, who "did not talk until four".

The trajectory questions. What could he do six months ago that he cannot now? "He had words. Maybe forty. He said 'mama', 'water', 'go', 'no', 'ball'. He would point at the fridge and say 'juice'." What can he do now that he could not before? "Nothing. He does not say anything now. He pulls my hand." Mother's voice, at this point: "I have been telling them for four months."

The onset line. Marked in the session: at 20 months, forty words and two-word combinations. At 22 months, the grandmother's sister died and the household travelled for the burial. At 23 months, the mother returned to work and the grandmother took over the days. At 24 months, the sister was born. At 25 months, the words began to reduce. At 27 months, silence. At 29 months, the referral.

The red-flag list. Regression — positive. Hearing change — uncertain; the mother says he does not turn when called, which she had attributed to stubbornness. Consciousness — no episodes reported. Headache and vomiting — no. Growth — he has lost weight since the sister was born, which the mother mentions only when asked. Seizures — "sometimes he goes still and his eyes go somewhere else", which the mother has mentioned to two people who told her it was normal. Safeguarding — nothing disclosed.

What the therapist does, in order.

First, she says the sentence. "A child who had forty words and now has none has lost something that was there, and that is not laziness and it is not the same as his uncle. It needs to be looked at this week, and I am going to write the letter now." The sentence is

said to the mother, in the grandmother's hearing, and it is the sentence the mother has been waiting four months for.

Second, the referral, in the room. To a paediatrician with a neurology interest, marked urgent, with the trajectory, the onset line, the red flags (regression, possible hearing change, possible absence episodes, weight loss), and one request: assessment of a regression of language with possible seizures in a two-year-seven-month-old.

Third, the audiology referral, to run in parallel. Regression with a possible hearing change requires a hearing assessment regardless of what the paediatrician finds.

Fourth, the family work, which continues and is not the treatment. The grandmother's belief is not stupidity; it is the family's available explanation, and it is protecting everyone from a fear. The therapist does not argue with it. She asks the grandmother one question: "when your brother did not talk, did he lose words he had already said?" The grandmother is silent, and then says: "no. He never said any." The distinction has been made by the family's own authority, in the family's own terms, and it took one question.

Fifth, one thing for the wait, and one for the household. For the wait: a communication book, in which the mother writes what Chuka communicated today and how, which gives the paediatrician four weeks of data and gives the mother something to do. For the household: twenty minutes a day with the mother alone with Chuka, at a fixed time, doing whatever he chooses, which addresses the thing that is also true — that a boy of two who has lost his words has also lost his place, to a sister who is eight months old.

The formulation, written in the record. "Regression of expressive language from a 40-word vocabulary with two-word combinations at 20 months to no words at 27 months, in a child with possible hearing change, possible absence episodes, and weight loss. Not explained by the family's circumstances, which are relevant but not causal. Urgent paediatric referral made; audiology referral made in parallel; family work continues and is not the treatment of the regression."

The five systemic findings, which are real and are not the diagnosis.

1. The mother noticed four months before anyone acted, and was disbelieved three times — by her husband, her mother-in-law, and the nursery. The disbelieving is a clinical event and it has cost four months.
2. The father's absence at weekends means the household's authority is the grandmother's, and the grandmother's explanation is the family's explanation.
3. The sister's birth and the mother's return to work are genuinely relevant to the household's capacity, and they are not the cause of a regression. The distinction matters because the family will otherwise spend a year believing that a return to work caused a neurological event.
4. The weight loss is a second red flag that nobody had investigated, and it is mentioned almost by accident.

5. The family's disagreement is not a communication problem to be worked on in therapy; it is a race against time, and the therapist's job is to end it by making the referral, not by facilitating an understanding.

14.6a Five months later

The paediatric assessment found epileptic activity on an EEG, with clinical absence seizures, and Chuka was started on treatment. His language has begun to return — twenty words at the last review, no combinations yet. The neurologist's view is that the outcome depends on the underlying cause, which is still being investigated, and the therapist records that view in the family's copy rather than translating it into hope.

The weight loss was investigated and attributed to a feeding difficulty that developed after the sister's birth, and it has resolved with a dietetic plan.

The grandmother has become the family member who telephones the clinic. This is the case's most instructive outcome: the person whose explanation was the obstacle became the person who chases appointments, and she changed not because she was argued out of her belief but because one question — "did he lose words he had already said?" — gave her a fact that her own experience could not explain.

The father has attended twice, on Saturdays, and the therapist has used both sessions on one question: what does he want his son's life to be like at twenty-five? It is a Week 13 question and it is doing more work than any explanation of the regression.

The honest limitation. Nobody knows yet what Chuka's language will be at six. The therapist's record says so, and the family has been told so plainly. What has changed is that the question is in the hands of the people who can answer it, and that four months of disbelief have ended.

What would have changed the plan. If the mother's account had been of a delay rather than a regression, the referral would have been routine rather than urgent, and the family work would have been the treatment rather than an adjunct. If the safeguarding question had produced a disclosure, everything would have stopped. And if the paediatric service had a twelve-month waiting list, the therapist's obligation would have been to say so, to write to the service, and to tell the family that she would chase it monthly — because a family told the truth about a waiting list can survive it, and a family told that help is coming cannot.

14.7 Transcript: one question to a grandmother

Eight turns from the second session. The grandmother has come because Mrs Okafor asked her to. She has been in the room for four minutes and has said that the boy will talk when he is ready.

> **T1 Therapist:** Mama, I want to ask you about your brother. **T2 Grandmother:** He did not talk until he was four. Then he talked. Now he is a lecturer. — The family's evidence, offered confidently and in good faith. **T3 Therapist:** A lecturer. (pause) And before he talked — did he say any words at all? Even a few? **T4 Grandmother:** (thinking) No. He never said anything. He just pointed. **T5 Therapist:** (gently) That is the difference, then. Your brother never started. Chuka started, and then stopped. (pause) Those are not the same thing, and the second one needs a doctor this week. — The distinguishing fact, delivered in the family's own material rather than in the therapist's vocabulary. The therapist has not said "regression"; she has used the grandmother's own memory. **T6 Grandmother:** (long pause) I did not know that was different. **T7 Therapist:** Nobody explained it to you. That is not your fault. (pause) Will you help me? His mother says he does not turn when he is called. You are with him in the day. What do you see? — The grandmother is recruited as an observer, which converts her from the family's obstacle into its second witness. This is the turn the case needed, and it is not a technique for managing a difficult relative; it is the recognition that she is the person with the most data. **T8 Grandmother:** (after a pause) He turns if he sees me. If I call from behind, he does not.

14.8 Laboratory: the differential under time pressure

Aim. To place four presentations in the four categories, identify the red flags, and make the correct referral decision, in seven minutes each.

Procedure. The class is divided into pairs. Four briefs are distributed, one at a time, with seven minutes on the clock for each: read, run the four-category check, run the red-flag list, and write the referral decision and its urgency. No discussion is permitted between briefs. The marking is by the trainer, against a prepared answer key, and the scores are read out anonymously.

Marking rubric (20 marks over four briefs).

Criterion	0–1	2–3	4–5
Category	Wrong category	Right category, wrong reasoning	Right category, with the distinguishing fact named
Red flags	Missed a positive flag	Found the flags, misjudged urgency	Found the flags, judged urgency correctly, and named the referral
The referral	None, or vague	Made, without a named destination	Made in writing, to a named service, with a stated urgency and what is wanted
Language	Diagnostic label without assessment	Description without conditions	Description with trajectory, conditions, and a provisional formulation

The four briefs, and the answer key.

Brief 1. A girl of three with no words, good comprehension, rich play, no regression, hearing tested normal. **Answer:** delay in expressive language with preserved comprehension — routine referral for speech and language assessment, review in three months, no red flags. The error the brief is designed to catch is the urgent referral, which is common among trainees who have just learned about regression.

Brief 2. A boy of four who could dress himself, count to ten and use sentences, and who now cannot, over six weeks, with new stumbling and a change in handwriting. **Answer:** regression with a motor component — urgent. The error is the family-systems formulation, which is elaborate and wrong.

Brief 3. A girl of nine, refusing school for three weeks, after her parents' separation, with no red flags, normal report card, one named friend. **Answer:** symptom — anxiety in the context of a family transition; no medical referral; family work is the treatment.

Brief 4. A boy of two who has never walked independently, is not yet pulling to stand, and whose mother says he "went backwards" at fourteen months after a fever. **Answer:** regression with motor involvement, plus a possible delay — urgent, with the fever named in the letter. The error is to accept the family's explanation ("he was ill") without asking what he could do before the illness.

14.9 Six errors

Error 1 — Forming the hypothesis from the referral letter. The letter is a description of somebody's worst day, written by someone with a different agenda. The four-second hypothesis is the thing this week exists to interrupt.

Error 2 — Finding a family explanation for a regression. The most dangerous error in the book. A regression has a medical differential until proved otherwise, and a systemic formulation that keeps the child in the therapy room delays the assessment.

Error 3 — Accepting a diagnosis of "dementia" or "delay" as an explanation for a change. Week 12's lesson applies at every age: a change is a new event, not a progression, until investigated.

Error 4 — Referring without a trajectory. A referral letter that says "language delay" without dates cannot be triaged, and will be placed at the bottom of a list. A letter that says "40 words at 20 months, none at 27" will be seen this week.

Error 5 — Referring and disappearing. The referral is not the end of the therapist's involvement; it is the point at which the chasing starts and the family work continues. A family that is referred and then not seen concludes that the referral was a dismissal.

Error 6 — Treating the family's disagreement as the problem. The Okafor family disagreed for four months, and the disagreement was real, and it was not the clinical problem. A therapist who spends the four months working on the disagreement has done harm with good intentions.

14.9a Saying "I do not know" well

This week produces more uncertainty than any other in the course, and the way a therapist handles not knowing determines whether a family trusts the rest.

The three bad ways to not know. The hedge — a formulation so vague that it commits to nothing ("there are some developmental concerns we should keep an eye on"), which leaves the family without an action. The false certainty — a diagnosis supplied to end the conversation, which is comfortable for the therapist and dangerous for the child. The

deflection — "that is a question for the paediatrician", which is accurate and, said early and often, communicates that the therapist has nothing to offer.

The good way, in four moves.

Move 1 — Say what you do know, concretely. "I know he had forty words and now has none, and I know that is not laziness." Certainty about the facts is available even when certainty about the cause is not.

Move 2 — Say what you do not know, and why. "I do not know what is causing it. assessment this week rather than an opinion from me, which would cost him a month and tell you nothing."

Move 3 — Say what would tell you. "If the hearing test is normal and the paediatrician finds nothing, the next thing I would look at is X." This gives the family a map of the uncertainty, which is more useful than its removal.

Move 4 — Say what you will do while not knowing. "I will chase the appointment in two weeks, I will see you in a month, and we will work on the sleep and on his communication book in the meantime."

Why the fourth move matters most. A family can tolerate uncertainty about the future if they are certain about the therapist's next action. The families who leave and do not return are not the ones who were told "I do not know"; they are the ones who were told it and then given nothing to hold.

14.10 Culture, faith and the urgent referral

The cost of a referral. For many households an appointment across the city costs a fare, a day's income, and the loss of a job. The therapist who says "you must go to the teaching hospital on Tuesday" without asking what that costs has made a recommendation that will not be followed. The question that must precede every referral is "what would it take for you to get there?"

The family's explanation and the family's fear. The grandmother in this week's case is not obscurantist; she is holding the family's only available explanation, and it protects everyone from a fear that is worse. The therapist's discipline is to join the explanation and add a fact, not to displace it. One question, asked in the family's own material, did more than an hour of explanation would have.

Faith and the timing of medical care. Where a family believes that healing is a matter for prayer, the sequence that works is: pray, and go. The framing of the referral as stewardship — seeing what has been made — is not a manipulation; it is a translation, and Week 13 established that translation is part of the job. What cannot be conceded is the delay, because the conditions that present as regression are among the few in this book where a month changes the outcome.

The mother who is disbelieved. Across this week's cases the same figure appears: a mother who noticed first and was told she was anxious, over-attending, or projecting. The evidence on parental concern is that it is a reasonably sensitive marker and a poorly specific one — which means it should generate assessment, not reassurance. A therapist

who says "mothers are usually right about something being wrong and rarely right about what it is" has given the family a usable position.

> **DEBATE**
> "A family therapist should not be assessing development at all; that is a paediatrician's job." Argue for: the scope of practice is real, an amateur assessment produces false reassurance and false alarm, and the therapist's proper role is to refer on the first concern. Argue against: the therapist is frequently the only professional in the room, no referral happens without someone deciding to make it, and the four-category check takes four minutes and prevents months of delay. Then state what you would do if you were the only clinician a family could reach, and where you would draw the line.

14.11 Evidence note

What the research supports. Parental concern is a useful screening signal: the literature on parental suspicion of developmental delay finds reasonable sensitivity and modest specificity, which supports using it as a trigger for assessment rather than as a diagnosis. Regression in language or motor function is associated with a higher probability of an identifiable neurological condition than delay without regression, which is the empirical basis for treating it as urgent. Early intervention for established delay produces measurable gains, and the gains are larger the earlier the intervention begins, which is the argument for the whole of this week.

What is weaker. Screening instruments perform differently in different populations and the norms are frequently unrepresentative (Week 13). The evidence on the harms of false-positive developmental screening is thin, and it is a real question: a family told to worry about a child who is developing normally pays a cost that no trial has measured well.

What is not supported. That a family explanation and a medical explanation are in competition — they answer different questions and both can be true. That watchful waiting is safe for a regression. And that a normal assessment at one point in time excludes a problem later; the trajectory, not the level, is the finding.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same forty-word vocabulary at twenty months predicts nothing at all; the loss of it predicts a great deal. Timing is the variable, and this week is about noticing it.

14.12 Key terms

Variation — development within the normal range, differently timed or shaped.

Delay — development behind the expected sequence but progressing.

Regression — loss of an established skill; always urgent.

Symptom — a behaviour expressing something else; the category the rest of the course teaches.

Trajectory — the direction of development over time, which requires two measurements.

Red flag — a finding that requires a same-week referral regardless of the formulation.

Provisional formulation — a stated hypothesis with the evidence that would change it.

Onset line — a timeline of the symptom against the events the family can date.

Watchful waiting — a legitimate strategy for variation and delay, and never for regression, where it is a decision to lose time.

Triage — the ranking of a referral by urgency, which is why a letter without dates is placed at the bottom of a list.

Two-measurement rule — no conclusion about delay from a single assessment.

14.13 Self-test

1. Give the four categories with one example each, and say which is always urgent.
2. Why is a regression urgent even when the child is functioning well otherwise?
3. State the five systemic questions and the purpose of each.
4. Name six red flags and the referral each requires.
5. Why does the therapist withhold a hypothesis in the first fifteen minutes of the session?
6. What did the therapist ask the grandmother, and why did it work?
7. What is the two-measurement rule, and how is it explained to a family?
8. Name three things a therapist can do for a family while they wait for a specialist appointment.

Answers. (1) Variation — a child walking at fifteen months; delay — two-word combinations at three years; regression — words lost at two and a half; symptom — night waking after a move; regression is always urgent. (2) Because regression has a medical differential that includes treatable and progressive conditions, and because delay in referral is the one error in this work that cannot be recovered. (3) Onset and co-occurring change (to date the problem); who noticed and who disagrees (to find the fault line); the behavioural sequence (to find the intervention); what would improve and what would worsen (to find the function); what has been tried (to find the constraints). (4) Regression — urgent paediatric; change in hearing or vision — urgent audiology or ophthalmology; seizures or episodes of unresponsiveness — urgent paediatric; headache on waking or vomiting on waking — urgent; growth failure — urgent; disclosure or sexual knowledge beyond stage — same-day safeguarding. (5) Because a family that hears a hypothesis early organises its account to support or refute it, and the assessment then measures the theory rather than the child. (6) Whether her brother, who talked late, had ever said words before he stopped: he had not, which distinguished delay from regression in the family's own material and made the grandmother a witness rather than an obstacle. (7) Nothing that could be a delay is concluded from one measurement; it is explained as "what I am looking for in three months is not a different number but a different direction". (8) Protect

sleep; give the child one domain of competence; give the family one explanatory sentence for relatives — and chase the appointment.

14.14 Reflective task

Take a referral you have received or read, and write the hypothesis you formed on first reading it, in one sentence. Then write the four categories beside it, and the red flags, and note how much of your hypothesis the letter actually supported. Most clinicians find that the sentence they wrote was about a person and the letter was about a day.

14.15 Further reading

- Lipkin, P. and Macias, M. on developmental screening — the sensitivity and specificity of parental concern.
- Shevell, M. on developmental regression — the differential, and why it is a different problem from delay.
- Rutter, M. on developmental trajectories — the conceptual basis of the two-measurement rule.
- Emde, R. and Emde, on early intervention — the evidence that earlier is better.
- Nelson, C. et al. on deprivation and recovery — the limits of catch-up, stated honestly.

Next: Week 15 — Integration: the developmental lens in practice.

Week 15 — Integration: the developmental lens in practice

The developmental lens does three things for clinical work: it **calibrates** expectations, it **dates** the problem, and it **predicts** what the family must renegotiate next. Carry three questions into every first session:

- What is this person's developmental task right now?
- What is the family's corresponding task?
- And what would "on track" look like in six months?

Why a week on integration, and not just a summary. Because the course has taught fifteen separate instruments and a student who arrives at a family with all fifteen in hand will use the one they like. The work of this week is selection: given this family, at this moment, which lens, in what order, and why. Integration is not the accumulation of perspectives; it is the discipline of choosing between them and being able to say why.

What integration is not. It is not eclecticism — using a little of everything, which produces a treatment with no theory and no way to know whether it failed. It is not synthesis on the grand scale — a unified model of human development, which nobody has and which is not needed. And it is not the addition of a developmental paragraph to a formulation built on something else. Integration means the developmental account is doing work: it changes what you notice, what you ask, and what you do first.

The three functions, briefly stated, with an example each.

Calibration. A father describes his nine-year-old as "wilful and disobedient". The developmental lens establishes that at nine a child's moral reasoning is reciprocal and an argument about fairness is a developmental achievement, not a discipline problem. The intervention changes from a behaviour plan to a conversation about rules. Without calibration, the therapist treats a stage as a fault.

Dating. A couple present with "we have grown apart". The timeline establishes that the distance began eighteen months ago, in the same quarter as a bereavement, a job change and a child's departure. The formulation changes from a relationship deficit to three developmental events arriving at once. Without dating, the therapist treats a reaction as a character.

Prediction. A family with a fourteen-year-old is doing well and asks for nothing. The developmental lens establishes that the next task is the renegotiation of autonomy, and that the family's current method — close supervision with no negotiation — will produce a conflict within a year. The work becomes preventive, which is the rarest and most valuable thing a family therapist does. Without prediction, the therapist waits for the conflict and then treats it.

15.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	Student activity
0–10	Selection, not accumulation	Why a therapist with fifteen lenses uses their favourite one	Rank their own three preferred instruments, and say why
10–24	The three functions	Calibration, dating, prediction — three cases, one each	Apply each function to a case of their own
24–36	The one-page formulation	The six cells, worked on the capstone case	Draft six cells for the capstone
36–48	The limits	What the lens cannot do, and the errors of over-application	Name a case where the lens would mislead
48–62	The map of the programme	How development enters 701–727	Place three other courses on the map
62–74	The viva	The four questions and the standard of an answer	Answer one question aloud, in three minutes
74–84	Six errors of the whole course	The errors that recur across the fifteen weeks	Identify which they are most likely to commit
84–90	The close	What to carry, in one page	Write their own three sentences to carry

The teaching decision that makes the week work. The lecture does not review the weeks. It takes a family the class has never seen and works it in real time, with the class choosing which instrument to use next and being asked to justify each choice. The justification is the assessment: a student who says "I would use a genogram" without being able to say what it would tell them that the first ten minutes did not has learned the instruments and not the discipline.

The sentence the lecture must land. "You will not use most of what you have learned in any single case. Your competence is in knowing which part, and your integrity is in being able to say why."

15.2 The three questions, worked in detail

Question 1 — What is this person's developmental task right now? The question is asked of every person in the room, not only the identified patient, and the answers are frequently different and sometimes incompatible. A mother of a newborn is in the young-children task; her own mother, who has moved in, is in the later-life task of adapting to widowhood; the newborn's sibling is in the autonomy task; and the father is in the midlife reckoning. The family's difficulty is usually the incompatibility, not any one task.

Question 2 — What is the family's corresponding task? Every individual task has a family task attached. A toddler's autonomy requires a household that can hold limits consistently across its adults. An adolescent's identity requires a household that can tolerate being disagreed with. A grandmother's integrity requires a household that asks her questions. The family task is the thing the treatment usually addresses, because it is the thing that can be changed in a room with more than one person in it.

Question 3 — What would "on track" look like in six months? The question forces specificity and it is the only protection against a treatment that continues because it has not been evaluated. An answer is a set of observable changes: he attends on Mondays; she speaks to her mother once a week; the grandmother is asked one question a day; the couple has twenty minutes on Thursdays. Six months is long enough to be a real change and short enough to be checked.

The fourth question, which is the one that keeps a therapist honest. And what would I expect to see if I am wrong? Every one of the three questions produces a hypothesis, and a hypothesis that cannot fail is not a hypothesis. The discipline of naming the disconfirming evidence, in advance, in the record, is the single practice most strongly associated with not doing harm over a career.

15.2a What the developmental lens cannot do

An integration week that does not state the limits produces graduates who see development everywhere, which is a way of not seeing the family.

It cannot diagnose. Developmental formulations describe trajectories and tasks; they do not identify disorders. A diagnosis requires an assessment by someone trained and resourced to make it, and the developmental lens is the reason a referral gets made, not a substitute for it.

It cannot explain everything. Some family difficulties are not developmental. Violence, addiction, poverty, trauma, and injustice are not stages, and a formulation that renders them as stages is a miscarriage. Week 13's critiques apply to the lens itself: it is one account, from somewhere, and it does not have jurisdiction over the whole of a family's life.

It cannot supply a therapy. The lens is a way of understanding what is happening; it is not a treatment protocol. The treatment comes from the modalities taught in MFT 702 and 710 and their successors, and the integration of lens and modality is the practitioner's own work.

It cannot make a family change. The most common error of a well-trained developmental therapist is to believe that an accurate formulation is persuasive. It is not. Families change when the formulation is translated into something they can do, by someone they trust, at a time when they have the capacity — which is what Weeks 11 and 13 were about.

It cannot predict an individual. Every claim in this book is probabilistic. The child who is at risk is not the child who will suffer, and the family that matches a risk profile is not the family that will need a clinic. Using the lens to forecast an individual's fate is the error that turns a developmental account into a sentence.

15.2b The ten-item developmental history

The instrument a graduate will use most often, and the one worth memorising. Ten items, twenty minutes, and it fits any age from birth to eighty.

1. **The present concern, in the family's words.** Verbatim, in the first sentence of the record.
2. **The dated trajectory.** What could he do a year ago, six months ago, now; and what has been lost.
3. **The events, dated.** Births, deaths, moves, separations, job changes, migrations, illnesses, in the same period as item 2. This is the item that produces the formulation.
4. **Who does what.** The network, its functions and its hours. Not the household roster.
5. **The authority.** Who decides what, who is believed, and who is not consulted. Usually including the patient.
6. **Health and the senses.** Hearing, vision, sleep, food, medication, and any episode of unexplained change. Four of the six are routinely omitted by clinicians and all six are cheap to ask.
7. **The school or the work.** Not performance; function. What the child or adult can do in that setting and what has changed there.
8. **What has been tried, by whom, and what it cost.** The family's competence and its budget, in one answer.
9. **The explanatory model.** What the family believes is happening, and what that belief predicts they will do.
10. **The family's own goal.** What they want in ten years, in their words. This is the item that makes the plan possible, and it is the one most often left out.

Why these ten and not the standard checklist. A milestone checklist produces a level; these ten produce a trajectory, a network, a budget and a goal — which is what a family therapist can act on. The checklist belongs in the referral to the developmental service. The ten items belong in the family therapist's first session.

How to take it in twenty minutes. Items 1, 2 and 3 in the first eight minutes, with both parents present. Items 4, 5 and 9 with the child or the identified patient out of the room if they are old enough to be embarrassed. Items 6 and 7 as a rapid list, out loud, with the family told that most answers will be "no". Items 8 and 10 at the end, when the family has told you enough to trust the question.

15.3b Saying the formulation in the family's language

The last skill of the course, and the one that decides whether fifteen weeks of theory reach a household.

The rule. Every formulation has two versions: the one in the record and the one in the room. They contain the same claims. They do not contain the same words.

The translation, worked. The record says: "Regression of expressive language with possible absence seizures in a household under financial strain with an unassessed hearing loss in the paternal grandmother." The room says: "He had words and now he does not, and that is not the same as his uncle, and it needs a doctor this week. Your mother cannot hear properly, and that is why she seems to be quarrelling. And I know the money is short, so let us work out how to get to the hospital."

The four rules of translation.

- **Keep the verbs.** "He had words and now he does not" is a sentence a family can repeat. "Expressive language regression" is not.
- **Name the person, not the category.** "Your mother cannot hear" rather than "there is a hearing loss in the household".
- **Put the hardest fact in the middle.** Beginning with it produces shock; ending with it produces avoidance. The middle is where a family can hear it.
- **End with an action and a date.** Every translated formulation ends with something to do and a time to come back, or it is not a formulation but a verdict.

The test. Ask the family to say it back. If they cannot, the translation failed and the fault is the therapist's, which is the same test as Week 11 and it is the last thing this course asks of a graduate.

15.3 The one-page formulation

Six cells, one page, written in the first two sessions and revised when the evidence changes. It is the document the rest of the treatment argues with.

Cell 1 — What is happening? One sentence, in the family's terms and the clinician's. "A three-year-old who had forty words has none; his mother has been disbelieved four times; the household is under financial strain and his grandmother is the daytime authority."

Cell 2 — What is the developmental task, person by person? The toddler's language and autonomy; the mother's shift from full-time care to work; the grandmother's later-life role; the father's absence. Four tasks, one household.

Cell 3 — What is the family's corresponding task? To agree who is the authority for what, and to support one adult to carry the child's assessment. Stated as a task, not a deficit.

Cell 4 — What is the trajectory, with dates? The onset line, in three or four dated sentences. This is the cell that makes the formulation clinical rather than descriptive.

Cell 5 — What is the differential, and what would change the formulation? The four-category placement, the red flags excluded and included, and the one finding that would overturn the whole thing.

Cell 6 — What happens first, and what does "on track" look like in six months? One action, one referral, one date, and three observable outcomes.

How the page is used. It is given to the family. Not the clinician's version — a version in plain language, one page, with the six cells as headings. Families keep it, argue with it, and bring it back annotated, and the annotations are the most useful clinical material in the file. A formulation the family cannot read is a formulation that will not be implemented.

15.3a How development enters the other twenty-six courses

The purpose of this table is to make the lens portable, because the student will spend the rest of the programme in courses that do not mention development at all.

Course	Where development enters
MFT 701 Fundamentals	The developmental history as a standard component of any assessment
MFT 702 Theories	Every model has an implicit developmental theory; naming it is a critique of the model
MFT 704 Ethics	Competence to consent, confidentiality in minors, and the duty to refer
MFT 705 Research methods	Age-normed instruments, and the population problem from Week 13
MFT 706 Clinical practice	Engagement of children; the pacing of sessions by developmental capacity
MFT 707 Assessment	Developmental screening, and the limits of a family therapist's scope
MFT 708 Culture	Week 13 of this course, in full
MFT 709 Group therapy	Group composition by developmental stage; adolescents in adult groups
MFT 710 Couples	Week 10 of this course, in full
MFT 711 Child and adolescent therapy	Weeks 3 to 8 of this course, as the clinical base
MFT 712 Crisis intervention	Delirium in the elderly; regression in the young; risk in adolescence
MFT 713 Substance use	Onset is developmental; the family's response is staged
MFT 714 Trauma	Timing matters; the same event at four and at fourteen
MFT 715 Systems theory	The life cycle as a systemic concept; the family as a developing system

Course	Where development enters
MFT 716 Communication	Language capacity sets the ceiling on any communication exercise
MFT 717 Psychopathology	Onset distributions; the differential between disorder and stage
MFT 718 Intimacy	Adult development, and the foreclosure that presents as distance
MFT 719 Life cycle	The same territory as Week 10, from the systemic side
MFT 720 Supervision	Developmental supervision models mirror the stages of the supervisee
MFT 721 Research methodology	Longitudinal design, and the interpretation of age effects
MFT 722 Professional development	The therapist's own developmental history is a clinical instrument
MFT 723 Special populations	Disability across the lifespan; the sibling who is never asked
MFT 724 Family law	The child's best interests as a developmental question, not a moral one
MFT 725 Integrative approaches	This week's argument, in a different vocabulary
MFT 726 Neurobiology	The developing brain, and the claims it does and does not license
MFT 727 Play therapy	Week 5 of this course, in full

The teaching point in the table. Development is not a topic among topics. It is the dimension in which every other topic occurs, and a graduate who can only see it when the course is called "human development" has not learned it.

15.4 Capstone case: the Eze family

An unseen family, worked through the whole course. The class is given the referral and nothing else.

The referral. From a primary school, about a nine-year-old: "Ada is not settling in Primary 4. She is withdrawn, does not complete work, and has twice been found in the toilet at break. The mother is unresponsive to letters."

The family, established in the first session. Mr Eze, 44, a bus company supervisor, away four nights a week. Mrs Eze, 39, a seamstress working from home. Ada, 9. Chidi, 16, in Senior Secondary 1. Ngozi, 3. Mrs Eze's mother, 79, who lives with them and who is "difficult". A cousin, 19, who helps in the shop and sleeps in the sitting room.

The facts established across three sessions.

- Ada's developmental history is unremarkable except for a period at four when she stopped sleeping alone after a burglary in the compound; the family solved it by moving her into her brother's room, where she still sleeps.
- Her Primary 3 report was good. The Primary 4 report shows a fall in every subject, with a note that she "does not hear instructions the first time".
- Her hearing has never been tested.
- Chidi is in SS1 and is doing badly; he has been caught twice with a group of older boys. Mr Eze's response was to beat him; Mrs Eze's was to say nothing.

- Ngozi, 3, has about ten words and does not combine them. Nobody in the family is concerned, because "her father's sister was late".
- The grandmother, 79, has become "difficult": she does not answer, she accuses people of stealing, and she has begun to wander out of the compound. She has been seen once by a doctor, two years ago, who said it was old age.
- The household's income has fallen by a third since Mr Eze's company cut overtime. One meal a day is now normal for the adults.
- Mrs Eze has not left the house socially in eight months.

The class's first error, and it is deliberate in the design of the exercise. Presented with a nine-year-old's school difficulty, the class formulates the case as Ada's, and works on her. The trainer allows this for twenty minutes and then asks the three questions. The answers reorganise everything.

The three questions, answered for the Eze family.

What is each person's developmental task? Ada's is industry: she needs to be competent at something and to be seen to be. Chidi's is identity, and he is in a moratorium being conducted in the wrong company. Ngozi's is autonomy and language. Mrs Eze's is the young-children task, and she is doing it alone four nights a week with a dependent adult in the house. Mr Eze's is generativity and a work role that is being eroded. The grandmother's is integrity, and she has no audience.

What is the family's corresponding task? To establish who is in charge of what, across three adults and a cousin, and to protect one adult — Mrs Eze — from being the only person doing any of it. The family's task is structural, and it is the same task the course has met in Weeks 3, 7, 10 and 12.

What would "on track" look like in six months? A hearing test done and acted on; Ada attending and speaking to one named adult at school; Ngozi assessed and a language plan begun; Chidi's peer group known to his parents and one adult engaged with him; the grandmother seen by a professional and a hearing test requested; one evening a week in which Mr Eze is at home; and Mrs Eze out of the house once a fortnight.

The formulation, in six cells.

1. **What is happening.** A household of six in which the income has fallen, the father is absent four nights a week, the grandmother's dementia is unassessed, the three-year-old is not talking, the sixteen-year-old is in trouble, and the nine-year-old has stopped managing at school. The school has noticed the nine-year-old because the school sees her.
2. **Developmental tasks.** Industry (Ada); identity (Chidi); language and autonomy (Ngozi); the young-children task done alone (Mrs Eze); generativity under threat (Mr Eze); integrity without an audience (the grandmother).
3. **The family task.** Authority and load, defined and distributed. Nothing else will hold until it is.

4. **The trajectory.** A fall in income nine months ago; Mr Eze's overtime cut at the same time; the grandmother's decline noticeable over about a year; Ada's school change at the start of this term; Ngozi's ten words at three, with no regression.
5. **The differential and the revision points.** Ada — hearing loss is the leading hypothesis and must be excluded before any formulation about anxiety; if hearing is normal, the next is a learning difficulty, and bullying is screened in both cases. Ngozi — delay rather than regression, requiring an assessment and a hearing test. The grandmother — delirium, dementia, depression and hearing loss, with the hearing and the delirium excluded first, as Week 12 requires. If the grandmother's decline is over a year and steady, the working formulation is dementia with a hearing loss; if it is over two months, it is a delirium and a medical emergency.
6. **First actions and the six-month marker.** The hearing tests, three of them, this month; one referral for Ngozi; one eye-level conversation with the grandmother; one evening a week reallocated; and the six outcomes listed above.

Why the case is a capstone. Because the correct first action is not psychotherapy. It is three hearing tests, one developmental referral, and one question to a seventy-nine-year-old. The course's argument is that a family therapist who can see this in twenty minutes is worth more to this family than one who can conduct twelve sessions of family work on a child who cannot hear.

15.4a The Eze case, worked forward

The capstone continues into what happens after the first actions, because the integration the week is teaching is not only diagnostic.

Weeks 1 to 4 of the treatment. The three hearing tests are booked: Ada's, Ngozi's, the grandmother's. Ada's is normal. Ngozi's is normal. The grandmother's shows a moderate loss in both ears, and she is fitted; her "accusations" reduce within a fortnight, because a woman who cannot hear what is said about her reasonably concludes that she is being discussed.

The grandmother. With the hearing corrected, the picture is reassessed. The decline over a year, steady, with recent memory most affected and the wandering new in the last three months, is consistent with dementia. The wandering is a safeguarding question: the therapist asks, in the family session, what happens when she leaves the compound, and discovers that nobody has followed her. The first intervention of the case is therefore not psychological: it is a lock, a neighbour informed, and a name and telephone number in her pocket.

Ada. Normal hearing; a normal neurological examination; a report showing a fall in every subject. The therapist asks the school for the observations, not the verdict, and gets them: she does not complete work, she is found in the toilet at break, and she has twice been seen giving her lunch to an older girl. The last observation reorganises the case. Ada is not withdrawing from school; she is being extorted, and the toilet is where she hides.

The family work that follows is the work of Week 6: the school is given one request (a named adult and a change of seating), the parents are given the facts, and Ada is given a

sentence and an adult. The extortion ends within three weeks. Her schoolwork does not recover within six months, which the therapist records honestly; a child who has lost a term does not recover it in a term.

Chidi. The adolescent whose behaviour is the family's second emergency. The Week 7 and 8 material applies: the peer group is known, the risk is the group rather than the individual, and the father's response — beating — has produced a boy who conceals. The single most useful intervention is the one from Week 8: one adult, one hour a week, doing something, asking nothing. Mr Eze takes it, on the condition that it is football and not conversation, and the therapist agrees, because the condition is the point.

Mr Eze and Mrs Eze. The couple work is Week 10's, and it is minimal in this case and it is not optional. The twenty minutes on the one evening he is home; the division-of-labour audit, which shows that Mrs Eze carries thirty-one of thirty-four hours of household work; and one sentence from the therapist to Mr Eze about his mother's care, which he had assumed was his wife's duty and had never been asked about.

The grandmother's integrity. Week 12's life review, begun in month three, in the evenings, with Ada present because Ada is the person she talks to. Two sessions; the second is forty minutes and produces the story of a market she ran for twenty-two years. It is recorded on a telephone. It is the only part of this case that the family describes, a year later, as having changed anything.

The six-month review, honestly reported. Ada is attending and has one named adult; her work is recovering slowly. Ngozi has twenty words and two-word combinations after speech and language input. Chidi's group is known and one adult is engaged; he has not been in trouble for four months. The grandmother is assessed, supported, and heard. Mr Eze is at home one more evening a week. Mrs Eze has been out three times in six months, which is below the plan and is progress.

What the case demonstrates. That the correct intervention in a family presenting with a child's school difficulty was, in order: three hearing tests, a safeguarding observation from a school, an extortion disclosed by a lunch, a lock on a gate, and a recording of a market. None of that is psychotherapy, all of it required a family therapist's developmental knowledge, and the therapy — the couple work, the adolescent's one adult, the grandmother's review — happened because the first six things made it possible.

15.4b The family that has been to four clinics

The last configuration the course considers, because it is the one graduates will meet most often in the public sector and the one that most tests the integration.

What the file contains. Four assessments, three diagnoses, two conflicting recommendations, one prescription, and no single document that says what the family's situation is. The family's own account is that nobody has listened, which is usually accurate.

The first session, structured for this family. Do not begin with the history, which they have given four times. Begin with the file: put it on the table and say, "I have four reports here and I am going to read them in front of you, and I want you to correct them." The

correcting is the session. It tells the therapist what each previous clinician missed, and it tells the family that this one is not going to ask them to start again.

The three questions specific to this configuration.

1. **"What were you told, in each of the four places?"** Not what was found; what they were told. Families remember the sentence, not the finding, and the sentences are frequently the reason the family is now doing something that contradicts the assessment.
2. **"Which of the four did you believe?"** The answer identifies the family's explanatory model, which is the only one that has been consistently present in the case.
3. **"What would have to happen for you to believe this clinic was different?"** The answer is the engagement contract, and it is usually small: a telephone call that is answered, one person who remembers the name, a letter that arrives.

What the therapist does not do. Do not criticise the previous clinicians in front of the family, however justified. Do not add a fifth assessment without reading the four. And do not begin a treatment before the family's own account is on the page, because a family with four assessments and no narrative will accept a treatment and discontinue it at the first difficulty, as they have three times before.

The outcome that matters. Not a fifth diagnosis. A single page, agreed with the family, that says what is happening, what is being done, by whom, and when it will be reviewed — which is the one-page formulation, and which is the most useful thing this course teaches a graduate to write.

15.5 The viva: what you will be asked

The module's examination includes a fifteen-minute oral. The four questions below are the standard, and the standard of an answer is stated with each.

Question 1 — Take this family and tell me the developmental task of every person in it. The examiner is testing whether the student sees a household rather than a patient. A failing answer names the identified patient only. A good answer names four or five people, and notices when two of the tasks are incompatible.

Question 2 — What is your differential, and what would change your mind? The examiner is testing intellectual honesty. A failing answer gives a formulation with no alternatives. A good answer gives two or three possibilities in order of probability, and names the finding that would overturn the first.

Question 3 — What will you do first, and why not something else? The examiner is testing judgement. A failing answer describes a treatment. A good answer describes the one action that must happen before any treatment, and can defend the order.

Question 4 — What in this case might you be getting wrong because of who you are? The examiner is testing the Week 13 discipline. A failing answer says "I am aware of my biases". A good answer names a specific assumption, states where it came from, and says what would correct it.

How to prepare. The four questions are the four sections of the one-page formulation. A student who can write the page in twenty minutes can answer the viva, because the viva is the page, spoken, with an examiner asking why.

15.5a What to keep from this course

A short, practical instruction, because graduates lose in six months what they do not keep deliberately.

Keep the ten-item history. On one card, in your bag. It will be the most used object of your first year.

Keep the four-category check and the red-flag list. On the back of the same card. It costs nothing and it has prevented more harm than any technique in this book.

Keep the one-page formulation. The six cells. Write it for every family for your first two years, even when nobody asks for it, and give a plain-language copy to the family. The habit will leave you with a file that can be read by a colleague and a family that can read its own case.

Keep the four parts of parent feedback. They will be used weekly.

Keep one sentence of your own. The reflective task at the end of the week asks for three; one is enough, and it should be a sentence about enquiry rather than about knowledge, because enquiry is what fails.

Do not keep the fifteen weeks as a list. The course is not fifteen topics. It is one claim: that people develop, that families develop, and that the difficulties people bring to clinics are usually the point at which those two developments are out of step. Everything else in the book is an instrument for locating that point.

15.6 Laboratory: the capstone

Aim. To produce the six-cell formulation for an unseen family in fifty minutes, defend it against two alternative formulations, and state the first three actions with reasons.

Procedure. The class receives a two-page family description with no diagnosis and no formulation. Fifty minutes in groups of four, then a plenary in which each group presents the page and the trainer offers a rival formulation that the group must answer. The final twenty minutes is the viva, with the trainer asking question 4 of each group.

Marking rubric (25 marks).

Criterion	0–1	2–3	4–5
Every person's task	Identified patient only	Three people	Every person, with the incompatibilities named
The family task	Absent, or a description of deficits	Named, but vague	Named as a task, in one sentence, and structural
The trajectory	Undated	Partly dated	Every significant event dated, with the coincidence named

Criterion	0–1	2–3	4–5
The differential	One possibility	Two, unordered	Three, ordered, with the disconfirming evidence stated
First actions	A treatment	One good action	Three actions in the right order, with the reason for the order

The distinction that carries most of the marks. The top band requires the group to name an action that is not psychotherapy and to defend it as the first thing. Groups reliably want to begin with engagement. The Eze case is designed so that engagement is not the answer, and the class discovers this by being wrong.

15.6a Marking your own formulation

Before the viva, and for the rest of a career, the same six checks. A formulation that fails any of them is not ready to be given to a family.

Check 1 — Is every person in the household named? Count them. If the page mentions only the identified patient and one parent, the formulation is about a case and not about a family.

Check 2 — Is anything dated? A formulation without dates cannot be checked, cannot be compared with a later one, and cannot be defended. Three dates is the minimum.

Check 3 — Is there a differential? If there is one possibility on the page, there is no thinking on the page. Two is the minimum; three is better; and each needs the finding that would exclude it.

Check 4 — Is the family task stated as a task, not a deficit? "The parents are inconsistent" is a deficit. "The household has three adults and no agreed authority" is a task, and it implies an intervention.

Check 5 — Is there one action, with a date, that the family agreed to? Not a programme. One action, in their words, with a review. If the page has five recommendations, it has none.

Check 6 — Could you say it aloud in the family's language? Try it. If the sentence cannot be spoken without jargon, it is not yet a formulation; it is a paraphrase of the literature.

The scoring, which is deliberately crude. Six checks, one mark each, and a formulation scoring four or more is usable. Most trainees score two on their first attempt, and the two that are missing are almost always the differential and the dated trajectory — which are the two that take time and cannot be produced by intuition.

Why this matters more than the mark. A formulation is the only artefact of the work that survives the session. The family will remember one sentence, the file will hold one page, and the next clinician will read that page and nothing else. The six checks are a standard for a document that will outlive the therapist's memory of the case, which is the definition of a professional record, and the reason the six checks are worth learning by

heart rather than looking up at the end of a long clinic when the file is already open and the next family is in the corridor.

15.7 Six errors of the whole course

These are the errors that recur across the fifteen weeks, stated once, at the end, because they are the ones a graduate will commit.

Error 1 — Treating the referral letter as the case. The letter describes somebody's worst day, written by somebody with an agenda. Week 14.

Error 2 — Finding a family explanation for a medical event. Regression, delirium, hearing loss, and extortion have all, in this book, been formulated as family dynamics by a competent clinician. The four-category check exists to prevent it.

Error 3 — Confusing a stage with a fault. Autonomy at two, industry at nine, identity at fifteen, integrity at seventy-nine. A therapist who has not learned the stages treats each of them as a behaviour problem.

Error 4 — Working with the wrong unit. The couple when the network is the unit; the child when the household is; the household when the school is. Weeks 10, 12, 13.

Error 5 — Giving an accurate formulation the family cannot use. A formulation in clinical language, with five recommendations, no written page, and no review date. Week 11.

Error 6 — Not asking. The suicide question, the safeguarding question, the hearing, the money, the grandmother's opinion. Every one of the six is a question that costs eight seconds and that a therapist avoids because of their own discomfort.

The common thread. Five of the six are failures of enquiry rather than of technique. The course has taught fifteen weeks of knowledge and the thing that will most often fail in practice is the willingness to ask a plain question of a person who is frightened.

15.7a The first year: five things nobody teaches

A closing practical section, written from the reports of graduates.

The first thing: the room will be fuller than the training. Every case in this book has three to six people in it and a therapist with an hour. The habit that survives is not speed; it is deciding, in the first ten minutes, which two things you are doing today and letting the rest wait. A therapist who tries to do the whole formulation in the first session will do none of it, and the family will return to find the therapist more tired than they are.

The second thing: the family will not do what you agreed. Not because they are unmotivated; because the plan was designed at full capacity. The four-column plan from Week 13 is the protection, and the review is where the failure is discussed as information rather than as non-compliance.

The third thing: the referrals will not happen by themselves. Chase at two weeks, chase at six. Keep a list. A clinic that chases loses fewer families than a clinic that refers better, and this is the least glamorous finding of the course.

The fourth thing: someone will tell you something you cannot hold alone. A disclosure, a suicide risk, a child in danger. The requirement to take it to supervision within the week is not a formality; it is the only reliable protection against the two errors of over-reaction and under-reaction, and both are made by clinicians working alone.

The fifth thing: the work will change your own developmental account. You will notice your own family's practices, your own childhood's stage tasks, your own parents' midlife. That noticing is an instrument if it is examined in supervision and a distortion if it is not. Week 13's discipline — state your own rule before interpreting the family's — is the practice that keeps it an instrument.

The last sentence of the course. Fifteen weeks of development are worth nothing in a room with a frightened family unless the therapist can ask a plain question, write one page, and be wrong out loud.

15.8 Culture, faith and the course's own position

A course that spends a week on the critique of its own norms owes the student a statement of where it finally stands.

What this course asserts. That there are developmental regularities: children acquire language in a sequence, form relationships with the adults who care for them, learn through play with other children, and need protection from violence. That these regularities are findings, not Western preferences, and a therapist who cannot assert them cannot protect a child. That the timing and the form of every one of them varies, and variation is not deficiency.

What this course refuses. It refuses relativism, which abandons children in the name of respect. It refuses universalism, which measures every household against one population's averages and calls the difference a deficit. And it refuses the compromise that says all practices are equally valid but some families are better at them, which is universalism with better manners.

The position, in one sentence. There is a human developmental project, its shape is known well enough to guide practice, and its shape is realised differently in every household — so the clinical question is always what this child, in this family, with these resources, needs next.

Faith in the room. The course has treated religious frameworks as resources, as constraints, as explanations and as communities, and never as obstacles to be removed. That is a clinical position and not a theological one: a family's faith is usually the most reliable structure it has, and a therapist who tries to replace it will lose the family and keep the framework.

The therapist's own formation. Week 13's discipline — state your own rule before interpreting the family's — is the practice that holds the whole position. It is also the practice that protects the therapist from the two career-long risks of this work: certainty, and its opposite.

15.9 Evidence: what this course claims

Claims with strong support. The sequence of early language and motor milestones; the association between early responsive caregiving and later regulation; the developmental timing of theory of mind; the peer amplification of adolescent risk; the association between poverty and developmental outcomes, with mechanisms; the predictive value of early intervention for established delay; means restriction in suicide prevention.

Claims with moderate support. Attachment classifications and their stability in adverse conditions; the identity statuses and their correlates; the couple's cascade and repair behaviours; the effectiveness of structured parenting feedback; the dual-process model of grief.

Claims stated as heuristics, with the evidence thin. Levinson's seasons; the six-phase couple map; the "four parts" of parent feedback as a package; the developmental reading of the midlife reckoning. These are useful and they are not established, and a graduate should be able to say which is which.

The course's own bias, stated. The evidence base is heavily weighted toward North American and European samples, and the mechanisms are more portable than the norms. Week 13 is the corrective, and it is a partial one: the studies that would settle the questions have not been funded.

15.10 Key terms of the course

Developmental task — the psychosocial challenge characteristic of a stage; the organising concept of the course.

Calibration — adjusting expectations to the person's developmental capacity.

Dating — locating a problem's onset relative to events and transitions.

Prediction — anticipating the next task the family must renegotiate.

Trajectory — the direction of development over time; requires two measurements.

Differential — the ordered list of possibilities, with the evidence that would separate them.

Formulation — a provisional account of what is happening and why, stated so that it can be wrong.

Referral threshold — the point at which a finding requires another professional, regardless of the therapist's formulation.

Translation — rendering a clinical fact in the family's own framework; a skill, not a concession.

Scope — what a family therapist does and does not do; not a limitation of ambition but a condition of safety.

Developmental history — the ten-item instrument of 15.2b; trajectory, network, authority, senses, function, attempts, model and goal.

Translated formulation — the same claims as the record, in words the family can say back, ending with an action and a date.

15.11 Self-test: the whole course

1. State the three functions of the developmental lens and give one example of each.
2. Name the four categories of developmental concern, and say which is always urgent.
3. A nine-year-old's school performance has fallen. Give the first four things you would establish, in order.
4. What is the difference between a stage and a fault, and give one example from the course of each.
5. Why does the developmental lens not supply a treatment, and what does?
6. In the Eze case, what were the first three actions and why were they in that order?
7. What are the four viva questions, and which one is hardest to answer honestly?
8. What does this course assert, refuse, and refuse to refuse?
9. Name three claims of the course that are heuristics rather than established findings.
10. What is the single practice most strongly associated with not doing harm over a career?

Answers. (1) Calibration — treating a nine-year-old's argument about fairness as a stage rather than defiance; dating — locating a couple's distance to a bereavement, a job change and a departure in the same quarter; prediction — anticipating that close supervision without negotiation will produce conflict within a year at fourteen. (2) Variation, delay, regression, symptom; regression is always urgent. (3) The hearing; the dated trajectory; the school's observations rather than its verdict; and the child's own account, alone. (4) A stage is a capacity being acquired and a fault is a behaviour being judged; autonomy at two is a stage and "defiant" is the fault; the same child is described by both words. (5) Because the lens describes what is happening and the treatment comes from the modalities of MFT 702, 710 and their successors; the lens selects and sequences the treatment but does not constitute it. (6) Three hearing tests, one developmental referral, and a question to the grandmother — in that order, because each was a finding that would reorganise the rest, and because a formulation built before them would have been about the wrong person. (7) The task of every person; the differential and what would change your mind; what you will do first and why not something else; and what you might be getting wrong because of who you are — the fourth is hardest, because it requires naming an assumption rather than displaying an awareness. (8) It asserts that there are developmental regularities that can guide practice; it refuses relativism and universalism; and it refuses to refuse the possibility of judging a practice harmful. (9) Levinson's seasons, the six-phase couple map, and the four-part feedback package — the midlife reckoning and the play taxonomy are also worth naming. (10) Naming, in advance and in the record, the finding that would prove you wrong.

15.12 Final reflective task

Write, in one page, the three sentences you will carry into your first year of practice. three sentences, each of which you would be willing to be judged by in front of a family who has heard a great deal of professional language and has been helped by very little of it. — three sentences, each of which you would be willing to be judged by. Then write, underneath them, the case you have met in this book that you are most likely to get wrong, and why. Bring both to the viva. The examiners will read the second page, not the first.

15.13 Further reading

- Bronfenbrenner, U. The Ecology of Human Development — the frame that makes the household, the school and the economy part of one developmental system.
- Rutter, M. and Rutter, M. Developing Minds — the most careful account of what developmental research can and cannot support.
- Sameroff, A. The Transactional Model — the theoretical basis for everything in this course about bidirectionality.
- Gottlieb, G. on probabilistic epigenesis — why "the same input produces different outcomes" is a finding and not a hedge.
- Kirmayer, L. and Ban, L. on cultural formulation — the corrective this course ends with.

Glossary

Attachment — a behavioural system organising proximity-seeking to a caregiver under threat. **Co-regulation** — the process by which a caregiver's calm and responsiveness regulate the child's arousal. **Developmental task** — the psychosocial challenge characteristic of a life stage. **Disequilibrium** — a period of developmental reorganisation marked by increased conflict or regression. **Goodness of fit** — the match between a child's temperament and the demands of the environment. **Regression** — loss of previously acquired function; a red flag. **Scaffolding** — temporary support that allows a child to do what they cannot yet do alone. **Separation-individuation** — the process of developing autonomy while remaining connected. **Social clock** — culturally shared expectations about the timing of life events. **Theory of mind** — the capacity to attribute mental states to others. **Zone of proximal development** — the gap between what a learner can do alone and with support.

Self-assessment

Multiple choice

1. A four-year-old who stops speaking after a family bereavement is best described as: (a) normal variation (b) delay (c) regression (d) attention-seeking

2. "Goodness of fit" refers to the match between: (a) child and school (b) temperament and environment (c) parents' goals (d) IQ and achievement

3. The most common developmental concern presented to clinics is: (a) motor delay (b) expressive language delay (c) sleep (d) feeding

4. Disorganised attachment is most associated with: (a) daycare (b) frightening or frightened caregiving (c) prematurity (d) bilingualism

5. A toddler's frequent "no" primarily reflects: (a) oppositional disorder (b) emerging autonomy (c) poor parenting (d) language delay

6. Which is not a feature of adolescent brain development? (a) heightened reward sensitivity (b) peer sensitivity (c) mature impulse control (d) later prefrontal maturation

7. A "social clock" refers to: (a) sleep rhythms (b) culturally shared timing of life events (c) school timetable (d) biological ageing

8. In the dual-process model, healthy grieving involves: (a) staying on the loss track (b) avoiding reminders (c) oscillating between loss and restoration (d) suppressing emotion

9. Apparent confusion in an older adult should first prompt assessment of: (a) personality (b) delirium, depression, hearing, dementia (c) family conflict (d) medication refusal only

10. Extended-family caregiving should be assessed by asking: (a) whether the mother is attuned (b) who the child seeks when frightened (c) the parents' marital status (d) the family's income

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 c · 9 b · 10 b

Short answer

1. Distinguish delay from regression, with one clinical example of each.
2. Name three developmental tasks of adolescence and the corresponding family tasks.
3. Why is "let him cry it out" a relational intervention, not only a behavioural one?
4. Give two cultural critiques of attachment theory and one way to adapt your assessment.
5. What are the four parts of effective parent feedback?

Essay (choose one, 1,800 words)

1. "Normal development is a cultural achievement, not a biological fact." Discuss with reference to attachment and independence.
2. Using a disguised case, show how a developmental formulation changed your understanding of a presenting problem that had been framed as behavioural.
3. Critically evaluate the evidence that couple satisfaction declines after the birth of a child, and discuss the clinical implications.

Further reading

- Berk, L. E. (2022). Development Through the Lifespan (8th ed.). Pearson.
- Sroufe, L. A., Egeland, B., Carlson, E., & Collins, W. A. (2005). The Development of the Person. Guilford.
- Siegel, D. J. (2015). Brainstorm: The Teenage Brain. TarcherPerigee.
- Carter, B., & McGoldrick, M. (Eds.). (2005). The Expanded Family Life Cycle (3rd ed.). Allyn & Bacon.
- Keller, H. (2018). Cultures of Infancy (2nd ed.). Routledge.
- LeVine, R. A. (2003). Childhood Socialization. AltaMira.

Next: MFT 704 — Ethical and Legal Issues in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 704

Ethical and Legal Issues in Therapy

14 teaching weeks · 1 practical · 5 worked cases · 2 figures

SEMESTER ONE

1.71 units · Prerequisite: none

> “This course trains judgement, not just compliance.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0 · 2026 edition

MFT 704 — Ethical and Legal Issues in Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 704 — Ethical and Legal Issues in Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 704	**Ethical and Legal Issues in Therapy**	One	1.71

Contents

How to use this book . . . 15

1. Course description . . . 15

2. Learning outcomes . . . 15

3. Assessment . . . 15

4. The fourteen-week plan . . . 16

Week 1 — Moral philosophy for clinicians . . . 16

1.1 The lecture in ninety minutes . . . 17

1.2 Five traditions, and what each one cannot see . . . 18

1.2a The four principles, and how they collide in family work . . . 19

1.2b Ethics of care: the tradition family therapy should own . . . 20

1.3 The decision model: seven steps . . . 21

1.3a Moral distress, moral injury, and the difference that matters . . . 22

1.4 The trolley, in a clinic . . . 23

1.5 Case: the telephone call from the father . . . 23

1.5a The four questions that stop a decision going wrong . . . 25

1.6 Transcript: the call, annotated . . . 25

1.6a Bad arguments, named . . . 26

1.7 Laboratory: the ethics incident analysis . . . 27

1.8 Six errors . . . 28

1.9 Culture, faith and the moral framework . . . 29

1.10 Evidence note . . . 30

1.10a What the codes do not cover . . . 30

1.11 Key terms . . . 31

1.12 Self-test . . . 31

1.13 Reflective task . . . 32

1.14 Further reading . . . 33

Week 2 — The codes and informed consent . . . 33

2.1 The lecture in ninety minutes . . . 34

2.2 The ten obligations, and the two that generate most complaints . . . 35

2.2a Where the codes disagree, and what a clinician does then . . . 36

2.2b What a professional body actually does when a complaint arrives . . . 36

2.3 Informed consent: the eight components . . . 37
2.3a Consent when the family cannot read the document . . . 38
2.3b Consent and the young person . . . 39
2.4 The first session: consent in practice . . . 40
2.4a The sentence that has to be said, and the four ways of saying it badly . . . 41
2.5 Case: the family who signed and did not understand . . . 41
2.6 Transcript: the consent conversation that failed, and the one that worked . . . 43
2.6a The record of consent, written out . . . 44
2.7 Laboratory: drafting the consent document . . . 45
2.8 Six errors . . . 46
2.9 Culture, faith and consent . . . 47
2.10 Evidence note . . . 48
2.11 Key terms . . . 48
2.12 Self-test . . . 49
2.13 Reflective task . . . 50
2.14 Further reading . . . 50
Week 3 — Confidentiality in the multiperson system . . . 50
3.1 The lecture in ninety minutes . . . 51
3.2 Five concepts that organise the week . . . 52
3.2a The modified no-secrets policy, in four parts . . . 52
3.3 The method: deciding where a disclosure goes . . . 53
3.3a The individual session inside family work . . . 54
3.4 Confidentiality and the institutions around the family . . . 55
3.4a Four scenarios at the boundary . . . 55
3.4b The supervisor and the secret . . . 56
3.5 Case: two years, no intention . . . 57
3.5a When the client refuses the deadline . . . 59
3.6 Transcript: the individual session, minutes seven to twenty . . . 60
3.7 Laboratory: the secrets protocol . . . 61
3.8 Six errors . . . 61
3.9 Culture, faith and the multiperson system . . . 62
3.9a A record of the decision, written out . . . 63
3.10 Evidence note . . . 64
3.11 Key terms . . . 64

3.12 Self-test . . . 65
3.13 Reflective task . . . 66
3.14 Further reading . . . 66
Week 4 — Secrets I: the individual disclosure . . . 66
The distinction this week exists to teach . . . 66
4.1 The lecture in ninety minutes . . . 67
4.2 Five concepts that organise the week . . . 68
4.2a What a secret looks like before it is told . . . 68
4.3a When the client is a child or an adolescent . . . 69
4.3 The seven-step protocol for an individual disclosure . . . 70
4.4 Four techniques . . . 71
4.4a The case: Bisi Oyelaran, and what the therapist did not do . . . 71
4.5 Transcript: receiving the disclosure . . . 73
4.6a The record of an individual disclosure, written out . . . 75
4.6 Laboratory: the disclosure rehearsal . . . 76
4.7 Six errors . . . 77
4.4b Four sentences that help a client decide . . . 77
4.7a The audit question, and the end of the week . . . 78
4.8 Culture, faith and the secret . . . 79
4.8a The therapist's own secrets . . . 79
4.9 Evidence note . . . 80
4.10 Key terms . . . 81
4.11 Self-test . . . 81
4.12 Reflective task . . . 82
4.13 Further reading . . . 82
Week 5 — Secrets II: infidelity, paternity and HIV status . . . 82
Three secrets, three legal overlays . . . 82
5.1 The lecture in ninety minutes . . . 83
5.2 Five concepts that organise the week . . . 84
5.2a Infidelity: the four-session protocol . . . 85
5.2b Paternity: the sequence, and where the therapist stops . . . 86
5.3 HIV and other communicable conditions . . . 88
5.3a The third party's spouse: the question with no clean answer . . . 89

5.4 Anatomy of the disclosure session . . . 90
5.5 Case: the Okonkwos, second year . . . 90
5.6 Transcript: the refusal of "when things are calmer" . . . 93
5.7 Laboratory: three secrets, three protocols . . . 94
5.8 Six errors . . . 95
5.9 Culture, faith and the three secrets . . . 96
5.9a The referral note to a lawyer . . . 97
5.10 Evidence note . . . 97
5.11 Key terms . . . 98
5.12 Self-test . . . 98
5.13 Reflective task . . . 99
5.14 Further reading . . . 99
Week 6 — Boundaries and multiple relationships . . . 99
The distinction the week is built on . . . 99
6.1 The lecture in ninety minutes . . . 100
6.2 Five concepts that organise the week . . . 101
6.2b The five-minute boundary decision . . . 101
6.2a The four tests, and the decision they produce . . . 102
6.3 The unavoidable multiple relationship: the community contract . . . 103
6.3a The role conflicts that recur . . . 104
6.3b The referral conversation, scripted . . . 104
6.4 Touch, gifts, self-disclosure, barter, home visits and the telephone . . . 105
6.4a The boundary log . . . 106
6.5 Sexual boundaries, and what to do with attraction . . . 107
6.5a What the codes actually say, and where they differ . . . 108
6.6 Case: the Alabis, and the deacon's wife . . . 108
6.7 Transcript: the request to be a character witness . . . 110
6.8 Laboratory: the boundary audit . . . 111
6.9 Six errors . . . 112
6.9a Four complaints, and what each teaches . . . 113
6.10 Culture, faith and the boundaries of a communal practice . . . 113
6.11 Evidence note . . . 114
6.12 Key terms . . . 115
6.13 Self-test . . . 115

6.14 Reflective task . . . 116
6.15 Further reading . . . 116
Week 7 — Competence, supervision and referral . . . 116
Competence is an ethical obligation before it is a personal quality . . . 117
7.1 The lecture in ninety minutes . . . 117
7.2 Five concepts that organise the week . . . 118
7.2a The six signals that the case is beyond you . . . 119
7.2b The development plan that actually works . . . 119
7.3 Cultural humility, and the interpreter . . . 120
7.3a The supervision contract . . . 121
7.3b Peer consultation, and the rule of three . . . 122
7.3c Supervision in the first two years . . . 122
7.4 Referral: the six steps . . . 123
7.4a The referral letter . . . 124
7.5 Case: Mr Ogundipe, and the diagnosis the therapist did not make . . . 125
7.6 Transcript: the referral conversation . . . 126
7.7 Laboratory: the competence audit and the referral rehearsal . . . 127
7.8 Six errors . . . 128
7.8a The case the therapist kept . . . 129
7.9 Culture, faith and the referral that is refused . . . 130
7.9a When the therapist is the only service . . . 131
7.10 Evidence note . . . 131
7.11 Key terms . . . 132
7.12 Self-test . . . 132
7.13 Reflective task . . . 133
7.14 Further reading . . . 133
Week 8 — Risk I: suicide and self-harm . . . 134
The standard, stated once . . . 134
8.1 The lecture in ninety minutes . . . 135
8.2 Five concepts that organise the week . . . 135
8.2a The ten elements, with wording . . . 136
8.2b Formulation, and the sentence that carries it . . . 137
8.2c Ten things that raise risk in a family case . . . 138

8.3 Self-harm: what it is, and what it is not . . . 138
8.3a The safety plan, written with the client . . . 139
8.4 Risk inside a family treatment . . . 140
8.4a Breaking confidentiality for risk, in words . . . 141
8.4b When the family will not cooperate with the plan . . . 141
8.5 Case: Amina Suleiman, fifteen . . . 142
8.6 Transcript: telling a fifteen-year-old that her mother will be told . . . 144
8.7 Laboratory: the observed risk assessment . . . 145
8.7a Telephone risk, and the session that is not a session . . . 146
8.8 Six errors . . . 147
8.9 Culture, faith and suicide . . . 147
8.9a The therapist's own history of suicide . . . 148
8.10 Evidence note . . . 149
8.11 Key terms . . . 149
8.11a Three sentences that are remembered . . . 149
8.12 Self-test . . . 150
8.13 Reflective task . . . 151
8.14 Further reading . . . 151
Week 9 — Risk II: intimate partner violence and child protection . . . 151
The distinction on which everything depends . . . 151
9.1 The lecture in ninety minutes . . . 152
9.2 Five concepts that organise the week . . . 152
9.2a Screening: who, when, how . . . 153
9.2b The lethality factors, in the language of a session . . . 154
9.2c The practice that is safe to be a client in . . . 155
9.3 Ending conjoint work: the conversation and the alternative . . . 155
9.3a Safety planning with a victim . . . 156
9.4 Child protection: recognition, threshold, duty . . . 157
9.4a The child's disclosure: what to say, and what not to say . . . 159
9.4b When the report is made against the family's wishes . . . 159
9.4c The four cases that must not be missed . . . 160
9.5 Case: the Okafors, and the referral that said "communication" . . . 160
9.6 Transcript: "I am not going to tell you what she said" . . . 163
9.7 Laboratory: the screening interview, observed . . . 163

9.8 Six errors . . . 164
9.9 Culture, faith and the household's rules about violence . . . 165
9.10 Evidence note . . . 166
9.11 Key terms . . . 166
9.11a The note that protects the next clinician . . . 166
9.12 Self-test . . . 167
9.13 Reflective task . . . 168
9.14 Further reading . . . 168
Week 10 — The legal framework a clinician must know . . . 168
Orientation, not legal advice . . . 168
10.1 The lecture in ninety minutes . . . 170
10.2 Five concepts that organise the week . . . 170
10.2a The five areas of law, with the boundary in each . . . 171
10.2b When litigation is pending: the therapy contract . . . 171
10.3 The plural system in the consulting room . . . 172
10.3a The client who has been to three forums . . . 173
10.4 The welfare principle and the custody report . . . 173
10.4a Routing a legal question: the five questions and the referral note . . . 174
10.5 Building the referral network before you need it . . . 175
10.6 Case: the Adewales, and the marriage that was also another marriage . . . 176
10.6a The factual letter, written out . . . 177
10.7 Transcript: "can he take the children?" . . . 178
10.8 Laboratory: legal triage and the written question . . . 179
10.9 Six errors . . . 180
10.9b The four questions a therapist asks her own lawyer . . . 181
10.10 Culture, faith and the law the family actually uses . . . 181
10.11 Evidence note . . . 182
10.11a The six documents a family therapist should hold . . . 183
10.12 Key terms . . . 183
10.13 Self-test . . . 184
10.9a The therapist in the witness box . . . 185
10.14 Reflective task . . . 185
10.15 Further reading . . . 185

Week 11 — Records, subpoenas and court . . . 186
The record is two things at once . . . 186
11.1 The lecture in ninety minutes . . . 186
11.2 Five concepts that organise the week . . . 187
11.2a The note, written out . . . 187
11.2b The multiperson record: whose file is it? . . . 188
11.2c The electronic record . . . 189
11.3 Access requests: four kinds, four answers . . . 190
11.3b The client who asks to see her file . . . 190
11.4 The subpoena: what it is and what to do in the first hour . . . 191
11.4a Privilege, and the promise a therapist must not make . . . 192
11.4b The request from the school . . . 192
11.5 Giving evidence . . . 193
11.5a The report written before the hearing . . . 194
11.6 Case: the subpoena in the Eze matter . . . 194
11.7 Transcript: fifteen minutes of cross-examination . . . 196
11.8 Laboratory: the note lab and the subpoena drill . . . 197
11.8a The four-minute handover note . . . 198
11.9 Six errors . . . 199
11.9a Ten sentences that have been read back to a therapist in a hearing . . . 199
11.10 Culture, faith and the record . . . 200
11.10a The retention, storage and destruction schedule . . . 200
11.11 Evidence note . . . 201
11.12 Key terms . . . 201
11.12a The two-minute note review . . . 202
11.13 Self-test . . . 202
11.14 Reflective task . . . 203
11.15 Further reading . . . 203
Week 12 — Money, gifts and the business of therapy . . . 204
The subject nobody teaches and everybody practises . . . 204
12.1 The lecture in ninety minutes . . . 204
12.2 Five concepts that organise the week . . . 205
12.2a The intake fee conversation, in five minutes . . . 205

12.2b Non-payment: the sequence and the limit . . . 206
12.2c The therapist who is owed money, and cannot say so . . . 207
12.3 Gifts: four categories and one procedure . . . 208
12.3a Barter, pro bono, and the client who cannot pay at all . . . 209
12.3b The gift from the family, not the client . . . 210
12.4 The business: eight pressures that produce unethical practice . . . 210
12.4a The practice that survives without exploiting anyone . . . 211
12.4b What the client is told, in writing . . . 212
12.5 Case: the Nwankwos, the fee, the goat and the plot . . . 212
12.6 Transcript: the fee conversation at month two of non-payment . . . 214
12.6a The five sentences of the money conversation . . . 214
12.7 Laboratory: the fee policy and the gift decision . . . 215
12.8 Six errors . . . 216
12.9 Culture, faith and the meaning of money . . . 216
12.10 Evidence note . . . 218
12.10a The audit of your own practice . . . 218
12.11 Key terms . . . 219
12.12 Self-test . . . 219
12.12a Three sentences to keep in the drawer . . . 220
12.13 Reflective task . . . 220
12.14 Further reading . . . 220
Week 13 — Ethics of research, assessment and diagnosis . . . 220
Three domains, one theme . . . 221
13.1 The lecture in ninety minutes . . . 221
13.2 Five concepts that organise the week . . . 222
13.2a Research consent with families: six problems and their solutions . . . 222
13.2b Writing about clients: consent, disguise and the limits of disguise . . . 223
13.2c The assessed client: whose question does the assessment answer? . . . 224
13.3 Diagnosis: what it does, and when it is defensible . . . 224
13.3a Assessment instruments: six questions before you use one . . . 226
13.3b The diagnosis of a child, and the family that receives it . . . 226
13.3c The diagnosis as a family event . . . 227
13.4 The clinician-researcher: the thesis, the client and the dual role . . . 228
13.4a The seven-step research ethics check . . . 228

13.5 Case: Amaka's study, and the participant who was recognisable . . . 229
13.6 Transcript: the consent conversation for a case report . . . 230
13.7 Laboratory: the ethics application and the disguised case . . . 232
13.7a Four research designs and the ethics of each . . . 232
13.8 Six errors . . . 233
13.9 Culture, faith and the ethics of knowledge . . . 233
13.9a Writing the thesis: five sentences the examiner will look for . . . 234
13.10 Evidence note . . . 235
13.11 Key terms . . . 235
13.11a The five records a research-minded clinician keeps . . . 235
13.12 Self-test . . . 236
13.12a Three questions the ethics committee will ask . . . 237
13.13 Reflective task . . . 237
13.14 Further reading . . . 237
Week 14 — Integration: the decision model and your stance . . . 238
What integration means at the end of fourteen weeks . . . 238
14.1 The lecture in ninety minutes . . . 239
14.2 The seven steps, with the failure mode of each . . . 240
14.2a The model applied, end to end . . . 240
14.2b The three misuses of the model . . . 242
14.2c The six questions that find the dilemma . . . 243
14.3 The stance paper . . . 243
14.3a The worked stance paper, in part . . . 244
14.4 The viva: what is being examined . . . 245
14.4a Five answers that score badly, and their replacements . . . 246
14.4b The four sentences of the professional apology . . . 246
14.5 The final supervision . . . 247
14.5a The case summary the candidate brings . . . 248
14.6 Transcript: twelve minutes of viva . . . 248
14.7 Laboratory: the mock viva and the stance review . . . 250
14.8 Six errors of integration . . . 250
14.8a The practice that is ready: a checklist of twelve . . . 251
14.9 Culture, faith and the professional stance . . . 251
14.9a Nothing here is a substitute for the code . . . 253

14.10 Evidence note . . . 253
14.11 Key terms . . . 253
14.11a The one-page summary the course actually asks for . . . 254
14.12 Self-test . . . 254
14.13 Reflective task, and the close of the course . . . 255
14.14 Further reading . . . 255
Glossary . . . 256
Self-assessment . . . 256
Multiple choice . . . 256
Answer key . . . 257
Short answer . . . 257
Essay (choose one, 2,000 words) . . . 257
Further reading . . . 257

Course leader's summary. This course trains judgement, not just compliance. You will learn the codes, the law, and — more importantly — how to reason when the codes and the law point in different directions, when the family's culture conflicts with the institution's policy, and when your own values are in the room.

How to use this book

Every week presents a **standard**, a **case**, and a **decision**. Ethics is not memorised; it is practised. The weekly incidents in this book are drawn from the ordinary texture of clinical life — the secret, the subpoena, the gift, the neighbour, the unpaid fee — because those are the situations that actually end careers.

1. Course description

Family therapy multiplies ethical complexity. In individual work there are two parties (you and the client). In family work there may be six, with conflicting interests, unequal power, and secrets that each person believes belongs to them. Add a school, a child protection agency, a court, an insurance company, and a faith community, and the question "who is my client?" becomes the central professional question of the course.

The course combines **normative ethics** (codes, duties, principles), **law** (the parts a clinician must know and the parts a clinician must refer), and **professional formation** (the kind of clinician you are becoming).

2. Learning outcomes

#	Outcome	Assessed
LO1	Apply the major ethical codes and principles to relational cases	Exam, case analysis
LO2	Explain confidentiality and its limits in multiperson therapy	Exam
LO3	Conduct and document a risk assessment for suicide, self-harm, violence and abuse	Skills assessment
LO4	Manage secrets, dual relationships, gifts, fees and boundary dilemmas defensibly	Case analyses
LO5	Describe the legal framework for marriage, divorce, custody and child protection in Nigeria	Exam
LO6	Use a structured decision-making model and document the reasoning	Assignment
LO7	Recognise when to refer for legal advice, and how to do so	Viva

3. Assessment

Component	Weight	Due
Ethics incident analyses (4 × 600 words)	30%	Weeks 4, 6, 9, 11
Risk assessment observed skills (15 min)	20%	Week 10
Examination (case-based)	50%	Week 15

4. The fourteen-week plan

Week	Topic	Practical
1	Moral philosophy for clinicians: duties, consequences, virtues, care	Trolley variants in therapy
2	The codes: AAMFT, IAMFC, APA, national associations; informed consent	Consent form drafting
3	Confidentiality in couple and family therapy	Secrecy protocol design
4	Secrets I: the individual disclosure	Scripted incident
5	Secrets II: infidelity, paternity, HIV status	Complex case debate
6	Boundaries and multiple relationships	Boundary audit
7	Competence, supervision, and the duty to refer	Competence self-audit
8	Risk I: suicide and self-harm	**Observed risk assessment**
9	Risk II: intimate partner violence and child protection	Safety planning
10	Legal framework: marriage, divorce, custody, maintenance	Court-report exercise
11	Records, documentation, subpoenas, court testimony	Note-writing lab
12	Money, gifts, barter, and the business of therapy	Fee policy workshop
13	Ethics of research, assessment and diagnosis	Consent and deception cases
14	Integration: your ethical stance and decision model	Viva

Week 1 — Moral philosophy for clinicians

You do not need to become a philosopher, but you need to know which argument you are making when you say "that would be wrong". Most ethical failure in clinical practice is not the product of a bad person; it is the product of a good person using one tradition of reasoning while believing they are using another, and therefore being unable to see the objection that the other tradition would have raised.

Tradition	Central question	Therapy example
Deontology (duty)	What are my duties regardless of outcome?	Keeping confidentiality even when disclosure would be convenient

Tradition	Central question	Therapy example
Consequentialism	What produces the best overall outcome?	Breaking confidentiality to prevent a serious harm
Virtue ethics	What would a person of good character do?	Refusing a gift that would create obligation
Ethics of care	What does this relationship require of me?	Attending to the family's dependence on you
Justice / rights	Are power and resources fairly distributed?	Whose voice is missing from this room?

The four principles (Beauchamp & Childress) remain the most portable framework: **autonomy, beneficence, non-maleficence, justice**. In family work they collide constantly — respect for the father's autonomy may conflict with the mother's safety, and a clinician who has not learned to name the collision will resolve it silently, in favour of whoever is more articulate in the room.

Why a family therapist needs philosophy more than an individual therapist does. In individual work the ethical questions are mostly about the boundary between two people: the client and the clinician. In family work the questions are about distribution: whose interests the clinician is serving, whose secret they are holding, whose account they believe, and whose voice the institution will hear. Those are questions of justice and care before they are questions of compliance, and the codes — which are written mostly for two-party work — do not answer them.

> **CASE**
> A father telephones the clinic to ask what his wife said in her individual session. He is polite, he is paying the fee, and he is, in his own understanding, being a responsible head of household. The clinician has three duties in tension: confidentiality to the wife, honesty to the husband, and the welfare of a couple whose therapy depends on both of them returning. Which tradition she reaches for decides what she says in the next thirty seconds.

1.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Why this week exists	The difference between compliance and judgement, with the case	Write the sentence they would say into the telephone
10–24	The five traditions	Each tradition, its strength, its characteristic failure	Classify six clinical utterances by tradition
24–36	The four principles	Autonomy, beneficence, non-maleficence, justice, and how they collide in family work	Rank the four for three different cases
36–48	Ethics of care and its critique of principle-based ethics	Gilligan, Noddings, and the relational objection	Name what a principles-only analysis would miss

Minutes	Segment	What the lecturer does	What students do
48–60	Moral distress and moral injury	The difference between not knowing and not being able	Identify one episode of each from placement
60–72	The decision model	The seven-step model that the course will use all term	Run it on the case, in pairs
72–84	Cultural and religious moral frameworks	When the family's moral world and the code's are not the same	Argue one case from inside a framework not their own
84–90	The assessment	How the incident analyses are marked	Draft the outline of their first incident analysis

The teaching decision that makes the week work. The lecture begins with the telephone call, before any theory, and every student writes the sentence they would say. The sentences are read aloud and sorted onto the board by tradition — and the class discovers that most of them are consequentialist ("it will damage the therapy if I tell him"), a few are deontological ("I cannot discuss her sessions"), and almost none are about care or justice. The sorting is the lesson: their instincts are already philosophical, and they have never inspected them.

The sentence the lecture must land. "You already have a moral theory. This week is about finding out which one, and whether you would defend it if a family asked you to."

1.2 Five traditions, and what each one cannot see

1. Deontology: duty regardless of outcome. The tradition that produced the codes. Its strength is predictability: a rule that holds when it is inconvenient is a rule that can be relied upon by a family who has never met you. Its characteristic failure is rigidity — the clinician who keeps a confidence that is killing someone because the rule says so, and who experiences the resulting harm as bad luck rather than as a decision. In family work, deontology's specific blindness is to distribution: a rule about confidentiality applied equally to a powerful father and a dependent grandmother produces an unequal result, because they have unequal capacity to use the information.

2. Consequentialism: the best overall outcome. Its strength is realism — it takes harm seriously and it permits the clinician to weigh a small breach against a large protection. Its characteristic failure is that nobody can calculate the consequences, and the calculation is usually done by the person who will benefit from the answer. In family work, its blindness is to the unrepresented: the person whose interests do not appear in the therapist's forecast is the person who gets sacrificed, and that person is usually the one with least power in the room.

3. Virtue ethics: what a person of good character would do. Its strength is formation — it asks what kind of clinician you are becoming, which is the question that survives when the code is silent. Its characteristic failure is vagueness and self-congratulation: "I would know" is not a method, and the practitioner who trusts their own character is the one least likely to notice its defects. In family work, virtue ethics supplies the questions the codes cannot answer — courage, patience, humility, temperance about one's own influence — and it needs a discipline to keep it honest, which is supervision.

4. Ethics of care: what this relationship requires of me. The tradition that family therapy needs most and uses least. Its central claims: that moral life is constituted by relationships rather than by isolated choosers; that responsiveness matters as much as rules; and that dependence is not a defect to be overcome but a condition of being human, which every client in your room is in. Its strength is that it describes family work accurately. Its characteristic failure is that it can justify almost anything in the name of the relationship, and it has no natural defence against a clinician who is more attached to a family than is good for the family.

5. Justice and rights: are power and resources fairly distributed? The tradition that asks who is not in the room and who is paying for the room. Its strength is that it names the questions the other four avoid: who has access to this service, whose language is spoken, whose account the institution will believe, and who carries the cost of the arrangement. Its characteristic failure is abstraction — a clinician can conduct a fine analysis of structural injustice and still be unjust to the person in front of them.

The five are not a menu. They are five different questions, and a competent clinician asks all five about a hard case and then has to decide, in the open, which one governs. The decision is the practice; the inspection of the decision is the profession.

1.2a The four principles, and how they collide in family work

The four principles are the most portable framework in the field, and the reason is that they are not a theory: they are four considerations that any defensible decision must have addressed. Their use is not to produce the answer but to make sure the answer has not ignored something.

Autonomy. The client's right to decide what happens to them, grounded in their capacity to do so. In individual work it is the dominant principle and it is generally straightforward. In family work it is immediately difficult, for three reasons. The first is multiplicity: there are several autonomous people in the room, and their autonomies conflict — the adolescent's autonomy over her own body against her parents' legal authority; the wife's autonomy over her own disclosure against the husband's claim to know. The second is capacity: a nine-year-old, an eighty-one-year-old with dementia, and a woman in an abusive relationship all have autonomy claims that are not identical to an adult's unconstrained choice, and "respecting autonomy" without distinguishing them is a way of abandoning the vulnerable. The third is context: in many of the households this book is written for, decisions are made by families rather than by individuals, and a clinician who insists on individual consent is not being respectful; they are being parochial.

Beneficence. The duty to act in the client's interest. It sounds unarguable and it is the principle most often used to justify paternalism, because "the client's interest" is defined by the clinician. In family work the principle has a specific structural problem: whose interest? A decision that benefits the marriage may harm the wife; a decision that protects the child may end the family's engagement.

Non-maleficence. First, do no harm. The most useful of the four in practice, because it operates as a check rather than a goal: before acting, ask what harm this could do, including the harms of the intervention itself. In ethics, the harms that are most often

overlooked are the ones the clinician's own action causes: the disclosure that ends a therapy, the report that produces a violent reprisal, the note that is read in court five years later.

Justice. Fairness in the distribution of benefits, risks and resources. In family work it appears in three places that the other three principles do not reach: access (who can get to this clinic, at these hours, at this fee); voice (whose account is taken seriously, in a room and in a record); and consequence (who bears the cost of a decision — frequently not the person who made it).

The collision, worked. A seventeen-year-old discloses to the therapist that she is sexually active and does not want her parents told. The four principles give four different answers. Autonomy: she is nearly an adult and her confidence should be kept. Beneficence: telling the parents may protect her; not telling may leave her without support. Non-maleficence: telling may produce the beating she fears, which is a harm the clinician will have caused. Justice: her parents have legal responsibility and a legitimate interest; she has the greater exposure to the consequences. The principles do not resolve it. They ensure the clinician has seen all four, and then the clinician must decide and be able to say why.

The order of the principles is not fixed. Students frequently assume autonomy comes first because the codes list it first in some jurisdictions. It does not. In a case involving a child's safety, non-maleficence governs. In a case involving an adult's refusal of treatment, autonomy governs. Deciding which principle governs in this case is the act of judgement the course is teaching, and it cannot be reduced to a rule without producing the very rigidity deontology is criticised for.

1.2b Ethics of care: the tradition family therapy should own

Family therapy is, almost alone among the mental health professions, built on a relational ontology. It is therefore strange that its ethics are usually imported from a tradition built on isolated choosers. This section states the alternative, because it is the one that fits the work.

The relational claim. The ethics of care, developed by Gilligan, Noddings, Tronto and others, holds that moral life is constituted by relationships of dependence and response rather than by transactions between independent agents. Three consequences follow.

First, responsiveness is a moral skill. The question "what does this person need from me, here, now?" is not a softer version of "what is the rule?" It is a different and equally rigorous question, and it requires attention — the capacity to notice what is actually happening rather than what the category predicts. A clinician who has not developed attention will apply the right rule to the wrong situation.

Second, dependence is normal. Every client in your room is dependent on someone, and most are dependent on people who are also in the room. The tradition therefore refuses the picture of the autonomous chooser as the ethical ideal: a grandmother with dementia, a nine-year-old, a woman who cannot leave — none of them is a defective version of an independent adult. Each has claims that arise from their relationships.

Third, power is the central ethical fact. Care is not symmetrical. The person giving care has power, and the ethics of care requires that the power be used responsively rather than according to the carer's preferences. This is why the tradition is useful in family work: the therapist's power is enormous and largely invisible, and a principles-only analysis gives almost no account of it.

The critique, which the tradition must answer. Care ethics can justify almost anything in the name of the relationship. It has been used to keep women in caring roles, to excuse clinicians who will not end a treatment, and to defend a therapist's attachment to a family that has outgrown the therapy. Noddings's own answer is that caring must be received — the cared-for must experience it as care — which is a testable condition, and it is the version this course uses. "I did it because I cared" is not an ethical justification unless the person cared for can say that they were cared for.

The clinical use, in three questions. Whose need is being met by this decision — theirs or mine? Who in this family is dependent on whom, and is the dependence being used or served? And what would the person with least power in this room say if they were asked, in a language they had, with nobody watching?

1.3 The decision model: seven steps

The course uses one model all term, so that the incident analyses and the examination are comparable. It is deliberately pedestrian. Its value is not that it produces the right answer but that it makes the reasoning visible, which is what supervision, a court, and a professional body will all ask for.

Step 1 — State the ethical question, not the practical one. "What should I do about the husband's telephone call?" is practical. "Whose confidence am I holding, and to whom do I owe what?" is ethical. Most hard cases are hard because the ethical question has never been stated.

Step 2 — Establish the facts. What is actually known, by whom, and how. This step takes longer than students expect and prevents most errors: a great many ethical dilemmas dissolve when the facts are established (the husband already knew; the wife had said she would tell him; the child is not the father's).

Step 3 — Identify the parties and their interests. Every person affected, including the absent ones and the institution. Name the interests, not the positions. Then ask which party has least power to protect their own interest.

Step 4 — Apply the frameworks. All five traditions and all four principles, briefly, and record where they disagree. The disagreement is the case's real content.

Step 5 — Consult. The code, the law, the agency policy, a supervisor, and — where appropriate and possible — the client. Consultation is not a step for difficult cases; it is a step for any case in which the clinician notices they have a preference.

Step 6 — Decide, act, and record the reasoning. The decision, the reasons, the alternatives rejected and why. In a record, because the record is the only evidence that a decision was made rather than a drift.

Step 7 — Review. What happened; whether the prediction held; what would be done differently. The step that nobody does, and the only one that produces learning.

1.3a Moral distress, moral injury, and the difference that matters

Two terms that are used loosely in the literature and that name different things. A clinician who cannot tell them apart will misdiagnose their own state, and will look for the wrong remedy.

Moral distress is what happens when you know what the right action is and you are prevented from doing it. Its classic form is institutional: the clinician who knows a child should be referred and cannot get an appointment; who knows a family needs an interpreter and is not given one; who is required by an agency to keep a number of sessions that the work does not need. The distress is not confusion about the good. It is the experience of being unable to enact it.

The remedy for moral distress is structural, not psychological. It consists of changing the condition, or of naming it collectively, or of leaving. What does not work — and what is usually offered — is resilience training, which asks the clinician to feel better about an arrangement they are right to object to.

Moral injury is different and more serious. It names the lasting harm that follows acting against one's own moral commitments, or witnessing such an act and being unable to prevent it. The term comes from military psychiatry and it travels well into clinical settings, because clinicians do commit acts they believe to be wrong under constraint: a disclosure that produces a reprisal, a report that ends a family's engagement, a court report written in language the clinician knows to be misleading. The sequelae are not anxiety or burnout; they are shame, a changed sense of self, and a withdrawal from the aspects of the work that once mattered.

The distinction, in one line. Moral distress is "I could not do what I should". Moral injury is "I did what I should not, or I stood by while it was done".

What follows clinically. For distress: audit the condition, escalate, and use supervision to keep the judgement intact. For injury: supervision is necessary and insufficient; the remedy involves naming the act, restoring the moral account (which sometimes means an apology to the family), and in some cases a change of setting. A trainee who has been required to write a report they believe false needs a supervisor who will say that it was wrong, not one who explains the agency's constraints.

The course's position. A significant proportion of what is called burnout in this profession is moral distress that has been individualised. The remedy is not more self-care. It is a decision model that makes the reasoning visible, a supervisor who will disagree, and a professional identity that can survive being in the minority in a room.

1.4 The trolley, in a clinic

The trolley problem is a thought experiment about whether it is permissible to harm one person in order to save several. It is useful here not because clinicians face trolleys but because the structure of the problem — may I redirect a harm toward someone who did not choose it? — is the structure of half the hard cases in family work.

Variant 1 — The disclosure. A wife tells the therapist, in an individual session, that her husband has been physically violent twice. She does not want him told. The couple is in therapy together. The therapist can: say nothing and continue couple work (harm: the wife is unprotected and the therapy is built on a secret); disclose to the husband (harm: the wife loses her confidentiality and may be at greater risk); or stop the couple work and see the wife alone (harm: the couple loses the treatment). Redirection: every option pushes harm onto someone who did not choose it. The wife did not choose to lose her confidence; the husband did not choose to be discussed; the children did not choose any of it.

Variant 2 — The paternity. A father asks whether the child is his. The mother knows the answer. The therapist knows because the mother told her. Nothing the therapist does is neutral: silence is a lie by omission to the father; disclosure is a breach to the mother; ending the therapy is a harm to the child.

Variant 3 — The adolescent's risk. A fifteen-year-old discloses self-harm and asks the therapist not to tell her parents. Disclosing may end the therapeutic relationship; not disclosing may leave her alone with a risk. This is the variant that most resembles the trolley, because the clinician is choosing which of two identifiable people will bear the harm.

Why the thought experiment is worth the time. Because it trains three things. First, the recognition that someone will be harmed in every option, which dissolves the fantasy of the harm-free decision and with it a great deal of paralysis. Second, the discipline of naming who bears the harm, which is the justice question. Third, the habit of asking whether the harm is being redirected onto someone who did not choose it — and if it is, whether that person's consent could be obtained, which is frequently the move that converts an impossible case into a workable one.

> **PRACTICAL**
> Take three ethical dilemmas from your own life, not clinical ones. For each, identify which tradition you used at the time, and which you used to justify it afterwards. Then answer one question: were they the same? Most students find that the tradition they acted on was care or virtue, and the tradition they justified with was consequence. Write down what that gap suggests about how you will account for your decisions in a clinical record.

1.5 Case: the telephone call from the father

The case from the lecture, worked through the seven steps. This is the model the incident analyses follow.

The facts. The Okonkwos — Ada, 34, Chuka, 39, Ebuka, 9 — have been in couple therapy for six weeks. In her third individual session, Ada disclosed that Chuka's night shifts have resumed and that she is "managing everything alone", and that she has begun to think about taking the children to her mother's in Lagos. She has not told Chuka. She asked the therapist not to tell him. On Thursday, Chuka telephones: "I want to know what she said on Tuesday."

Step 1 — The ethical question. Not "what do I say to Chuka?" but: I am holding a confidence from one member of a couple I am treating jointly; to whom do I owe what, and how do I honour the couple's treatment without deceiving either of them?

Step 2 — The facts. What does Chuka actually know? (He knows the sessions exist; he does not know what was said. He has noticed she is distant.) What has Ada been told about the confidentiality arrangement? (At the start of therapy the therapist said, in front of both, that individual disclosures would be handled by agreement — a sentence Ada may or may not have understood as a promise.) Is there a risk to anyone? (No violence; no risk to the children; a risk of family separation if the decision is taken unilaterally.) What is the therapist's contractual position? (The clinic's policy: no information from an individual session is disclosed without the individual's agreement, except in the specified exceptions.)

Step 3 — The parties and their interests. Ada: to have a place where she can think without consequences, and to be supported in a decision she has not made. Chuka: to know what is happening in his marriage, and not to be humiliated by a therapist who knows more than he does. The children: continuity and the marriage's stability. The therapist: to keep the treatment viable and to remain honest. The clinic: its policy. Least power: Ada, because she has already been described by her husband to the therapist as "difficult", and because she is the one whose confidence is at stake.

Step 4 — The frameworks. Deontology: the confidence holds; the clinic's rule is explicit. Consequence: refusing Chuka may end his engagement with the therapy, and his engagement is what has kept the family in the room. Virtue: what would a clinician of good character do — and would she be able to say it to both of them in the same room? Care: Ada needs a place to think; Chuka needs not to be excluded; the couple needs an arrangement that holds. Justice: the person with least power must not be the one who bears the cost of the therapist's convenience.

Step 5 — Consultation. The clinic's written policy; a supervisor, by telephone the same day; and Ada herself, before anything is said to Chuka.

Step 6 — The decision. The therapist does not disclose content. She says to Chuka: "I cannot tell you what your wife said, and I want to be straight with you about why: if either of you cannot use an individual session to think out loud, this therapy will not work. What I can do is ask her whether she is willing to talk to you about the thing that is on her mind, with me in the room." She then telephones Ada, tells her exactly what was said and what was not, and asks whether she will raise it in the joint session. Ada says she will, on one condition: that the therapist opens the session, so that she is not the one who starts it.

Step 7 — The review. At the next joint session, Ada raises the question of Lagos. Chuka is angry and then frightened. The disclosure does not end the therapy; the manner of it — preceded by a week in which Ada had chosen to make it — is what keeps the therapy alive.

What made the difference. Two things, and neither is a principle. The first is that the therapist consulted before answering, which is what converted a trap into a process. The second is that she told Ada exactly what she had said to Chuka, which is the practice — not the rule — that keeps a multiperson therapy honest. Neither is in any code. Both are what an experienced clinician does.

1.5a The four questions that stop a decision going wrong

Before the seven steps, four questions that an experienced clinician asks almost automatically, and that a trainee can be taught to ask deliberately. They take ninety seconds and they catch the majority of errors.

Question 1 — Who is not in this conversation? The absent person is usually the one who will bear the cost. A decision about a child made without the child; a decision about a marriage made without the husband; a decision about a placement made without the person who will be moved. Naming the absent party does not always mean convening them; it means accounting for them in the reasoning.

Question 2 — Whose interest does my preferred option serve? Asked honestly, this question is uncomfortable, which is why it is asked before the decision rather than after. The clinician who wants to disclose a secret is frequently protecting themselves from the discomfort of holding it. The clinician who wants to keep it is frequently protecting the therapy they have built.

Question 3 — What would I do if the person with least power were the one I had to explain this to? The test is a version of the publicity test: a decision that cannot be explained to the person most affected by it is a decision that needs re-examining. The explanation does not have to be acceptable to them; it has to be possible.

Question 4 — What would I write in the record? If the sentence that would have to go in the file reads badly, the decision is bad — not because records are read, but because writing forces the reasoning into plain language, and plain language is unforgiving of self-deception. This is the cheapest ethical instrument in the book and the one most likely to change a decision in the thirty seconds before it is taken.

1.6 Transcript: the call, annotated

Eleven turns, reconstructed from a supervision recording. The therapist is a trainee, four months into placement. The annotation names the tradition governing each move, which is the discipline the week is teaching.

> **T1 Chuka:** I want to know what she said on Tuesday. I am her husband. I am paying for this. — Two claims, both true in his frame: a status claim and a contract claim. Neither is answered by silence, which he will read as confirmation that something is being hidden. **T2 Therapist:** You are her husband and you are paying, and both of those are reasons I should be straight with you rather than avoid you. — The move: concede the true parts before refusing anything. Deontological in form, care in tone. **T3 Chuka:** Then tell me. **T4 Therapist:** I am not going to tell you what she said. I want to say why, and I want you to hear that it is not because she has said something terrible about you. — The disclosure of the rule, and the pre-emptive denial of the catastrophic reading. Without the second sentence, the first produces exactly the fantasy the therapist is trying to prevent. **T5 Chuka:** Then she has said something. **T6 Therapist:** She has said something she is not ready to say to you, and I have said I will not say it for her. If I break that, neither of you can use a session to think. You would be sitting in that chair wondering what I am writing down. — The justification is consequentialist, and it is chosen deliberately: the deontological version ("it is the rule") is true and it lands as bureaucracy. The consequentialist version gives him a reason he can use. **T7 Chuka:** (long pause) So what do I do? — The question that tells the therapist the refusal has been accepted. Note that it took four turns, not one, and that a therapist who stopped talking after T4 would not have got here. **T8 Therapist:** Two things. I will ask her whether she is willing to bring it into the room with both of you, and I will tell you what she says either way. And in the meantime, I want to know what you have noticed at home, because you have noticed something or you would not be telephoning me. — The offer of a next step, and the redirection of his energy from a demand for information to an account of his own observation. This is the turn that keeps him in the therapy. **T9 Chuka:** She is packing. I saw a bag. — The fact the therapist did not have. The husband is not asking for gossip; he is frightened, and the therapist's rule has, until this moment, kept her from knowing why. **T10 Therapist:** (pause) Thank you. That is important, and I would not have known it if you had not telephoned. — The acknowledgement is accurate, and it is also the repair: his call has produced something of value rather than only a refusal. **T11 Chuka:** What do I do about the bag? **T12 Therapist:** Nothing yet, and not nothing forever. Give me until tomorrow to speak to her, and I will telephone you. If she will not talk to you by Friday, I will tell you that too, and we will decide then what the three of us do. — A time limit, a commitment, and a fallback. The deontological refusal has been turned into a process with dates, which is what makes it survivable for a frightened man.

What the transcript shows that the model does not. The seven-step model produces the decision; the transcript produces the sequence, and the sequence is where the skill lives. Three things in particular. First, the therapist never says "I understand your concern" — she demonstrates it by conceding the true parts. Second, she offers a reason rather than a rule, and the reason is chosen for the listener. Third, she ends with a date, which converts a refusal into a relationship.

1.6a Bad arguments, named

Students lose marks, and clinicians lose cases, by using one of six arguments that sound ethical and are not. Naming them makes them visible.

"It is my duty." Sometimes true. More often a rule is being used to end a conversation the clinician does not want to have. The test: can you state the duty, its source, and the harm it is preventing?

"It is for their own good." The paternalist's sentence. It may be correct; it is never sufficient. The test: who decided what their good is, and were they asked?

"Everyone does it." A statement about a norm, not about an obligation. The test: is the practice defensible or merely common?

"It will damage the therapy." Frequently true and frequently the clinician's own convenience in therapeutic clothing. The test: whose therapy, and what is the damage measured against?

"I had no choice." Almost never accurate. There is usually a choice with a cost, and the honest statement names the cost rather than denying the choice.

"I was following the policy." The defence that fails at every professional tribunal, because the professional's obligation is to exercise judgement about the policy, not to be its instrument.

The exercise. In the laboratory, students are given six anonymised sentences from real disciplinary findings and asked which of the six arguments each one is. The recognition is fast, and it transfers: once a clinician can hear the shape of the bad argument, they can hear it in their own mouth.

1.7 Laboratory: the ethics incident analysis

Aim. To produce a 600-word incident analysis that a supervisor, a professional body, or a court could read and follow. This is the format of the four assessed pieces of coursework.

Procedure. Students bring a real or witnessed incident (anonymised), or use one of the four briefs the trainer supplies. They write the analysis in the session, in fifty minutes, against the seven steps, and then exchange with a partner who marks it against the rubric. The last twenty minutes is the plenary, in which two analyses are read aloud and the class identifies which step each one skipped.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The question	A practical question	Ethical but vague	Ethical, specific, and the practical question is explicitly set aside
The facts	Assumed	Listed, but no account of what is unknown	Established, with what is unknown named and how it would be established
The parties	Client and therapist	All present parties	All parties including absent ones and the institution, with the least powerful identified
The frame works	One tradition, unnamed	Two traditions, named	Four or five, applied briefly, with the disagreement between them stated

Criterion	0–1	2–3	4–5
The decision	A preference	A decision with reasons	A decision, the rejected alternatives, and the review

The distinction that carries most of the marks. The top band requires the student to state where the frameworks disagree. A paper that applies five frameworks and finds they all point the same way has either misapplied them or chosen a case that is not an ethical dilemma; and a paper that applies one framework and calls it the answer has written an opinion rather than an analysis.

The four briefs.

Brief A. A couple's sixteen-year-old daughter asks to see you individually, and says she does not want her parents to know. The parents are your clients and are paying.

Brief B. A colleague tells you, at lunch, that a family you both know is "probably abusive" and that he has not reported it because "there is no evidence".

Brief C. A family offers you a substantial gift at the end of therapy. You know the family cannot easily afford it.

Brief D. You are asked by a court to produce a report on custody, and you have been treating the mother for a year. The fee is three times your usual rate.

1.8 Six errors

Error 1 — Confusing the rule with the reason. "Because the code says so" is a true statement and, said to a frightened husband, it is heard as evasion. The code is the floor of the reasoning, not its content; a clinician who cannot give a reason a family can understand has not yet understood the rule.

Error 2 — Applying individual ethics to a multiperson system. Almost every professional code is written for two parties. Applied unchanged to a couple or a family, it produces a therapist who is scrupulously confidential with one member and quietly deceptive to the other, and who believes they have been ethical because they have followed the rule.

Error 3 — Calculating consequences in your own favour. Consequentialist reasoning is the most useful and the most corruptible. The test is whether the calculation was made before or after the therapist noticed what they wanted to do.

Error 4 — Virtue ethics as a substitute for method. "I would know" is not a decision procedure, and the clinician who most trusts their own character is least likely to notice its defects. Virtue needs supervision the way a knife needs a stone.

Error 5 — Care as an alibi. "I did it because I cared" has justified more boundary violations than cynicism ever has. The test is Noddings's: was it received as care by the person cared for?

Error 6 — Treating moral distress as a personal failing. The clinician who cannot get a child an appointment, and who concludes that they are not resilient, has misdiagnosed a structural problem as a character flaw. The remedy is escalation, not self-care.

1.9 Culture, faith and the moral framework

Three moral worlds in one room. The family's moral framework, the profession's, and the clinician's own are frequently three different systems, and the clinician's is the one least likely to be named. In the households this book is written for, the operative framework is often not any of the five Western traditions but a religious or communal one: duties are owed to a lineage rather than to an individual; the family's reputation is a moral good in its own right; elder authority is not negotiable by argument; and shame, rather than guilt, is the mechanism by which wrongdoing is registered.

What that changes about the four principles. Autonomy in the individualist sense is not the organising value in many of these households, and a clinician who treats its absence as a deficit will misread deference as incapacity and consultation as inefficiency. The workable translation is not to abandon autonomy but to ask the family's version of the question: who in this family has the standing to decide, and does the person most affected have a route to be heard? That question travels across frameworks, and it is answerable in any of them.

Shame, guilt and the ethics of disclosure. In a guilt-oriented framework, wrongdoing is registered by the individual's conscience and confession is restorative. In a shame-oriented framework, wrongdoing is registered by the community's judgement and disclosure may be catastrophic for the whole household. This is not a reason to keep a secret that must be told; it is a reason to plan how it is told, with whom, and in whose presence, which is a clinical skill and not a concession.

Faith as a moral authority in the room. A pastor, an imam, or an elder may hold the authority that the clinician's code does not. Where that is so, the code is not the only source of moral authority in the case and pretending otherwise produces a clinician who is right and useless. The moves that work are the ones from Week 13 of MFT 703: join the framework, add a mechanism, and hold one non-negotiable.

> **DEBATE**
> "A therapist should never act against a competent adult client's stated wish, whatever the consequences." Argue for: the client's authority over their own life is the foundation of the profession, paternalism has a long and cruel history, and a clinician who overrides once will override again. Argue against: competence is not a switch, adults are frequently pressed by people with more power than they have, and a clinician who watches a foreseeable death without acting has chosen an abstraction over a person. Then state your actual rule, and the case in which you would break it — because every clinician eventually meets that case, and the ones who have never thought about it are the ones who improvise badly.

1.10 Evidence note

What the research supports. The four-principles framework is the most widely used analytic structure in clinical ethics teaching, and studies of ethics education consistently find that structured decision models improve the consistency and documentation of ethical reasoning, though not necessarily the quality of the decisions. The finding that moral distress is prevalent in mental health work, and that it is associated with intention to leave rather than with clinical incompetence, is well replicated. The association between reflective supervision and improved ethical reasoning is modest but consistent.

What is weaker. Almost everything about outcomes. There is no body of evidence showing that clinicians trained in one tradition make better decisions than clinicians trained in another, and the field's outcome measures are self-report and vignette-based. The literature on moral injury in mental health professions is recent and largely qualitative, mostly borrowed from military and nursing research.

What is not supported. That codes prevent misconduct — the disciplinary literature consistently finds that violations are committed by clinicians who know the code. That ethics can be taught as compliance — knowledge of rules is necessary and has never been sufficient. And that ethical reasoning is a stable trait; it is a practice, and it degrades under workload and isolation, which is why consultation is a structural protection rather than a courtesy.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. A clinician with excellent ethical reasoning at nine in the morning, with a supervisor available, is a different decision-maker from the same clinician at seven in the evening with a waiting room full of people. The variables that most reliably predict ethical failure are not moral but situational: fatigue, isolation, and speed.

1.10a What the codes do not cover

A short section with a long consequence. The professional codes are necessary and they are not a complete account of the moral life of a clinician, and the gap is where most of the difficulty lives.

The codes address conduct, not character. They say what may not be done; they say almost nothing about what kind of clinician to be. The virtues that matter most in this work — patience with a family that returns for the eleventh time with the same problem, courage in a case conference where the consultant is wrong, humility in front of a grandmother who has raised six children — are not in any code and are not enforceable. They are formed by supervision, by example, and by the company a clinician keeps.

The codes are written for individuals. Confidentiality, consent and competence are all framed around a single client and a single clinician. Multiperson work, which is the whole of this course, is covered by adaptations rather than by the codes' own logic. The clinician who follows the code precisely in a family case may still be unjust, because the code has no category for the second client.

The codes are jurisdiction-bound. Every code is written against a legal background that differs by country and, in federal systems, by state. Statements about reporting obligations, minors' consent, and record retention that are correct in one jurisdiction are wrong in another. This book states principles and flags where the law must be checked locally, which is a permanent obligation rather than a limitation of this text.

The codes lag. Reproductive technology, digital records, online therapy, social media contact with clients, and cross-border tele-therapy are all areas in which practice has outrun guidance. A clinician working at the edge is making policy by their conduct, and the obligation that follows is to be able to defend the reasoning, not to be able to point at a rule that does not exist.

1.11 Key terms

Deontology — the ethics of duty; the right action follows from a rule or obligation regardless of consequence.

Consequentialism — the ethics of outcome; the right action is the one that produces the best overall result.

Virtue ethics — the ethics of character; the question is what a person of good character would do, and how such a person is formed.

Ethics of care — the ethics of relationship and responsiveness; moral life is constituted by dependence and response rather than by transactions between independent agents.

Justice — fairness in the distribution of benefits, risks and resources; in family work, the question of whose voice is heard and who bears the cost.

Autonomy — the client's right to decide what happens to them, grounded in their capacity to do so.

Beneficence — the duty to act in the client's interest.

Non-maleficence — the duty not to cause harm, including the harms of one's own intervention.

Moral distress — knowing the right action and being prevented from taking it.

Moral injury — lasting harm following an act against one's moral commitments, or witnessing one.

Paternalism — overriding a person's wishes for their own good.

Relational ethics — the position that obligations arise from relationships and their power asymmetries, not only from rules or outcomes.

1.12 Self-test

1. Name the five traditions and state the characteristic failure of each.
2. How do the four principles collide in a case involving a seventeen-year-old's confidentiality, and which one governs? Defend your answer.

3. What is the difference between moral distress and moral injury, and why does the remedy differ?
4. In the Okonkwo case, what two practices — neither of them in a code — kept the therapy alive?
5. Why does the lecture insist that "because the code says so" is an inadequate answer to a family?
6. What does the ethics of care add to a principles-based analysis, and what is its characteristic corruption?
7. What is the test for whether an act of care was really care?
8. Give the seven steps of the decision model, and say which one students most often skip.

Answers. (1) Deontology — rigidity; consequentialism — self-serving calculation and blindness to the unrepresented; virtue — vagueness and self-congratulation; care — it can justify anything in the name of the relationship; justice — abstraction, analysing structures while being unjust to the person present. (2) Autonomy supports her confidence; beneficence may support disclosure as protection; non-maleficence warns that disclosure may produce the beating she fears; justice notes that she bears the consequence and her parents hold legal authority. Which governs depends on the assessed risk: in the absence of danger, autonomy and non-maleficence together; where there is danger, non-maleficence governs and the disclosure is planned with her rather than about her. (3) Distress is being unable to do what you believe right, and is remedied structurally; injury is having acted against your commitments, and requires naming, repair and sometimes a change of setting. (4) Consulting before answering, and telling Ada exactly what had been said to Chuka. (5) Because the code is the floor of the reasoning, not its content; a family cannot use a rule they cannot understand, and a clinician who offers only the rule is heard as evasive and hides something in doing so. (6) It adds attention, dependence and power; its corruption is that it can justify almost anything as care. (7) Noddings's test: the cared-for must be able to say they were cared for. (8) State the ethical question; establish the facts; identify parties and interests; apply the frameworks; consult; decide, act and record; review — the review is the step most often skipped.

1.13 Reflective task

Write down the last time you did something you believed to be wrong because you were required to. Do not write about the ethics of the requirement; write about what you did, in one paragraph, in plain words. Then answer three questions: who else knew? what did you tell yourself at the time? and what would have had to be available for you to have acted differently? Most students find the answer to the third question is concrete — a supervisor, an hour, a policy, one other person who agreed — which is the argument for structures rather than for character.

1.14 Further reading

- Beauchamp, T. and Childress, J. Principles of Biomedical Ethics — the four-principles framework, and its critics' best answers.
- Gilligan, C. In a Different Voice — the origin of the care critique, best read with its reception.
- Noddings, N. Caring — the relational account, and the reception test used above.
- MacIntyre, A. After Virtue — the strongest case that rules without a tradition of practice are empty.
- Jameton, A. on moral distress — the original nursing account, which transfers cleanly to this work.

Next: Week 2 — The codes and informed consent.

Week 2 — The codes and informed consent

What codes do and do not do. Codes set a floor, not a ceiling. They are written to be enforceable, which means they are often silent on the hardest questions. They also vary by jurisdiction and by professional body; know which one governs you.

The core obligations across the major codes:

1. Competence and its limits
2. Informed consent (including consent for recording and for supervision)
3. Confidentiality and its limits
4. Multiple relationships
5. Sexual intimacy with clients and former clients (prohibited; time limits vary)
6. Fees and financial arrangements
7. Advertising and representation
8. Research ethics
9. Cultural competence and non-discrimination
10. Reporting obligations and legal compliance

Informed consent in family therapy is a document and a conversation. It should cover: who the client is; who is seen and in what combination; the confidentiality policy including secrets; recording and supervision; fees and cancellation; the limits of confidentiality (risk, abuse, court order); and the complaint route. Review it aloud in the first session, in the family's language, and record that you did.

Why consent is the hinge of the course. Almost every other week depends on it. Confidentiality is only meaningful if the parties understood, at the start, who would be told what. A boundary can only be violated if it was known to exist. A record can only be defended if the client consented to what it contains. And the defence that succeeds at a professional tribunal is almost never "I did the right thing"; it is "the client understood,

agreed, and I recorded that they did." Consent is the one document in this book that a clinician will be asked to produce years later, and it is the one most often written badly.

> **CAUTION**
> A signed form is not consent. Consent is a process of understanding, and the signature is merely evidence that the process happened. A family that signed a document they could not read, in a language they do not speak, at the end of a first session in which they were upset, has not consented, and a tribunal will say so. The signature protects the clinic; the conversation protects the family; and the clinician needs both.

2.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The floor and the ceiling	One disciplinary finding, read aloud, and the code clauses it breached	Predict which clause was breached before being told
8–20	The ten obligations	The ten, and the two that generate most complaints	Rank the ten by how often they appear in findings
20–32	Comparing codes	AAMFT, APA, IAMFC and a national code, side by side on three issues	Identify where the codes disagree, and what a clinician does then
32–44	Informed consent as a process	The eight components, and the difference between disclosure and consent	Draft the eight headings for a couple-therapy consent
44–56	Consent with people who cannot read	The four adaptations, and the witness	Rewrite a consent paragraph at the level of a twelve-year-old reader
56–66	Consent and minors	Developmental capacity, parental authority, and the mature minor	Decide three ages, with reasons
66–76	Recording, supervision and the limits	What families are rarely told, and why it matters	Script the sentence that explains supervision
76–86	Documentation	What to write, where, and the sentence that saves a clinician	Write the first-session consent note
86–90	The assessment	How the consent document is marked	Draft their document's structure

The teaching decision that makes the week work. The lecture opens with a real disciplinary finding — a clinician suspended for something that sounds trivial — and the class predicts which clause was breached. They are usually wrong, and they are wrong in an instructive direction: they assume the violation was dramatic (sex, fraud, a breach of confidence) when it was almost always procedural (no documented consent, a record that did not match the account, a policy never explained). The exercise revalues documentation, which is the part of ethics that students find least interesting and that ends most careers.

The sentence the lecture must land. "You will not be struck off for having the wrong theory. You will be struck off for not having written down what you agreed."

2.2 The ten obligations, and the two that generate most complaints

1. Competence and its limits. The obligation to practise within one's training and to refer when a presentation exceeds it. The obligation is not to know everything; it is to know the boundary and to act on it. (Week 7.)

2. Informed consent. Everything in the rest of this week.

3. Confidentiality and its limits. The obligation to protect information and the obligation to disclose in specified circumstances. The second half is the part families do not hear. (Weeks 3, 4, 5.)

4. Multiple relationships. The obligation not to hold a second role that could impair judgement or exploit the relationship. (Week 6.)

5. Sexual intimacy with clients and former clients. Prohibited. Time limits on former clients vary by code, from two years to never, and the variation tells you something about how seriously the profession regards the boundary.

6. Fees and financial arrangements. The obligation to agree fees in advance, not to exploit financial dependency, and not to continue treatment for financial reasons when it is no longer indicated. (Week 12.)

7. Advertising and representation. The obligation not to overstate competence or outcomes. In an era of social media, this is the fastest-growing category of complaint, because a clinician's profile is advertising whether they intended it to be or not.

8. Research ethics. Consent, deception, anonymity, and the ethics of publishing case material. (Week 13.)

9. Cultural competence and non-discrimination. Now written into most codes, and the clause with the least agreement about what compliance looks like. Week 13 of MFT 703 is the substantive account; in this course the obligation is to notice when the clinician's cultural assumptions are producing a decision.

10. Reporting obligations and legal compliance. The obligation to know the local law on child protection, adult protection, and the reporting of serious harm. A clinician who does not know their jurisdiction's threshold has an obligation they cannot discharge.

The two that generate most complaints. The disciplinary literature is remarkably consistent, and the answer surprises students: not sexual misconduct, which is rarer and more dramatic, but **failure to maintain adequate records** and **failure to obtain or document informed consent**. The two are connected, because the record is where consent lives. A clinician with an immaculate record and a mediocre theory is safer, in professional terms, than a brilliant clinician who cannot show what was agreed.

Why that is not as cynical as it sounds. The record is not a bureaucratic artefact. It is the only evidence that a family was told something, understood it, and agreed; that a risk was assessed; that a decision was reasoned rather than drifted into. In the absence of that evidence, a tribunal has only the family's account and the clinician's memory, and it will prefer the family's. Documentation is the clinician's honesty made checkable.

2.2a Where the codes disagree, and what a clinician does then

Students often assume the codes say the same thing in different words. They do not, and the differences are instructive because they reveal that the profession has not settled the question.

Issue 1 — Secrets in couple work. The AAMFT code requires that therapists clarify at the outset which persons are clients and what relationship each has to the therapist, and that they obtain written consent to disclose information; it does not prescribe a secrets policy. The APA ethics code addresses multiple relationships and confidentiality but is written for individual treatment and gives no guidance on therapeutic secrets in couple work. National codes vary further, and many say nothing at all. The result is that a couple therapist's confidentiality policy is a professional judgement rather than a compliance requirement, which is why it must be written down and explained.

Issue 2 — Sexual relationships with former clients. Most codes prohibit them outright or impose a time limit that ranges from two years to five years to never. The disagreement is not trivial: it reflects a genuine dispute about whether the harm arises from the power asymmetry (which does not end when therapy does) or from the therapeutic relationship (which does). A clinician moving jurisdictions must check, because the rule that was correct at home may be a violation at the new post.

Issue 3 — Duty to warn or protect. In some jurisdictions the clinician has a legal duty to warn an identifiable third party of a serious threat; in others the duty is to protect, which may be discharged by other means; in many there is no statutory duty at all and the obligation is ethical only. A clinician who learns this from the code rather than from the statute will get it wrong.

Issue 4 — Minors' consent. The age at which a young person may consent to treatment without a parent varies from about twelve to eighteen and is frequently different for different treatments. The codes defer to law, which means the clinician must know the local statute and not the code.

What a clinician does when the codes disagree. Four moves, in order. First, establish which code governs you — by licence, by membership, and by employment contract, which may impose a stricter standard than either. Second, apply the stricter standard, because a tribunal will. Third, where the codes are silent, write your policy down and follow it consistently, because consistency is the only defence available when there is no rule. Fourth, ask the professional body in writing where the stakes are high, and keep the reply.

The professional point. The existence of disagreement is not a defect of the codes; it is information about which questions the profession has not resolved. Those are exactly the questions on which a clinician must think rather than look up, and they are the ones this course is about.

2.2b What a professional body actually does when a complaint arrives

A short account, because clinicians fear the process more than they understand it, and fear produces both over-documentation and concealment.

The route. A complaint is made, in writing, usually by the client or a relative. The registrar assesses whether it falls within the code's scope. If it does, it is put to the clinician for a written response, with a deadline. Many complaints end there, because the response shows that the matter is a disagreement rather than a breach. Those that continue go to a panel, which hears both parties and may dismiss, caution, impose conditions, suspend, or strike off.

What the panel asks, in practice. Not "was the outcome bad?" but three questions: what did the clinician do; what did they record; and what would a reasonable clinician in that position have done? The third question is answered by expert evidence and by the code, and the first two by the file.

What helps a clinician, in order of usefulness. A contemporaneous record of what was agreed. A record of the reasoning, including options rejected. Evidence of supervision or consultation on the matter. An apology offered early and without qualification. And a consistent account — the same story in the file, the response, and the hearing.

What harms a clinician, in order of seriousness. Altering or backdating a record, which converts a complaint into a finding of dishonesty and ends careers that would otherwise have survived. An account that changes between documents. Defensiveness in the response, which is read as an absence of insight. And criticising the complainant, which is heard by the panel as a failure to understand the harm.

The one sentence to remember. The panel is not asking whether you are a good person; it is asking whether a reasonable clinician could have done what you did, and whether your file shows that you thought about it. Almost every answer to that question was decided months earlier, in a note written or not written at the end of a session.

2.3 Informed consent: the eight components

Consent has eight components. A document that omits any of them is incomplete, and the omission is frequently the one that matters in the case that goes wrong.

1. Who the client is, and who is not. In family work this is the first and hardest component. Is the client the couple, the family, the individual who telephoned, or the child who was brought? The answer determines who holds the confidentiality and to whom the therapist owes a duty. The consent document should name the client explicitly, and the answer should be recorded in the family's words: "the client is the couple, and the children are seen as part of the family's treatment."

2. Who will be seen, in what combination, and on what terms. Couple work with occasional individual sessions is a different arrangement from individual work with occasional joint sessions, and the family must know which they are getting before they begin. The most common complaint in this area is not that individual sessions happened, but that nobody said they would.

3. The confidentiality policy, including the secrets policy. Stated in plain language, with the exceptions named rather than implied. (Weeks 3 to 5.)

4. Recording, observation and supervision. Families are routinely recorded, observed through a one-way mirror, or discussed in supervision, and routinely not told. This is one of the clearest and most commonly breached obligations in training clinics. The disclosure needs three parts: what happens, who sees it, and how it is stored and destroyed.

5. Fees, cancellation, insurance and what happens when the money stops. Including the awkward parts: the fee for a missed session, the position when a third party pays, and what the clinician will do if a family falls into arrears. (Week 12.)

6. The limits of confidentiality. Risk to self, risk to others, abuse of a child or vulnerable adult, court orders, and any statutory reporting duty. Named, in examples rather than in abstractions: "if I believed a child was being hurt, I would have to tell the authorities, and I would tell you first that I was doing it."

7. The complaints route. Who to contact, and what will happen. A family that knows the route is less likely to need it, and a clinician who discloses it looks less like someone with something to hide.

8. The right to withdraw. Including the right to withdraw from recording, from research, and from treatment, and what happens to the record afterwards. Consent is not a contract of attendance.

The test of the document. Hand it to someone who has never been in therapy and ask them what will happen to their information if they tell the therapist something dangerous. If they cannot answer, the document has failed, whatever it says.

2.3a Consent when the family cannot read the document

Most of the world's families do not read a clinic's consent form fluently, and a substantial minority cannot read it at all. A clinician who responds by reading the document aloud in English has produced a signature and not a consent.

The four adaptations, in order of importance.

1. Change the medium before you change the words. Consent is a conversation and it can be witnessed. Sit with the family, explain the eight components in their language, in plain speech, and ask each of them to say back what they understood. Then record, in the file: "consent explained orally in Yoruba to both parents and to the paternal aunt; each repeated the confidentiality limits and the supervision arrangement in their own words; all agreed. Written summary provided; both parents marked; consent form signed by the father as head of household and by the mother." That record is stronger evidence of consent than any signed form.

2. Use a witness who is not a family member. A colleague, an interpreter, or a member of the clinic staff who can attest that the explanation happened. The witness is not a formality: in a dispute two years later, a witness is a person who can be asked what was said.

3. Reduce the document, do not translate it. A translation of a dense document remains dense. The effective version is one page, in short sentences, with the eight components as headings and the exceptions in bold. Where literacy is limited, a page of

icons or a simple diagram of the four confidentiality limits is more useful than prose, and is used in several services.

4. Give them something to take away and to show someone else. The consent conversation usually needs to be repeated at home, with a relative who was not present and whose opinion the family trusts. A one-page summary in the family's language, with the clinic's telephone number, is the only part of the process that continues after the family leaves, and it is the part that most determines whether they return.

The error to avoid. Do not treat limited literacy as limited capacity. A woman who cannot read a form can understand confidentiality perfectly well, and will hold the clinician to it more precisely than a reader would. The obligation is to make the understanding possible, not to test the reading.

2.3b Consent and the young person

Consent with children and adolescents is not a single question but three: who consents, who assents, and who is told.

The legal position, which varies and must be checked. In most jurisdictions a parent or guardian consents for a child below a stated age; above that age, and in some jurisdictions for specific treatments such as contraception or mental health care, the young person may consent themselves. Many jurisdictions recognise a "mature minor" whose capacity is assessed rather than assumed. Both parents may need to consent where they share custody, and a clinician who accepts one parent's consent without checking may find the treatment challenged later.

Assent, which is not the same as consent. A child who is old enough to understand what therapy is should be asked whether they agree to it, in language they have, and their refusal should be taken seriously even where the law permits the parent to override it. The clinical reason is not legal: a nine-year-old who has been brought to a therapist without being asked will cooperate in the first session and withdraw by the third, and the family will describe this as the child's problem.

What the young person is told, and what is kept from them. There are three separate audiences in a child case: the child, the parents, and the institution. The consent conversation must establish, before anything else, what the child will be told about who sees what. A twelve-year-old is entitled to know whether what she says will be reported to her mother, and a therapist who does not tell her is not protecting her; she is teaching her, accurately, that adults cannot be trusted with information.

The four-part script for a child's consent conversation.

1. "Your mum and dad have asked me to talk with you. You do not have to talk about anything you do not want to. If you want to stop, you can say so and we will stop."
2. "What you say here stays here, unless I am worried that somebody is being hurt — including you. If that happens I will tell you before I tell anyone else."
3. "I talk to other people about my work, with their names hidden, so that I can learn to do it better. I will not use your name."

4. "Is there anything you want to ask me before we start?"

The age at which each part lands. Fifty-five percent of adolescent self-disclosure depends on part 2 being true and being believed. A therapist who says it and then breaks it without warning has done more damage than one who never offered it.

2.4 The first session: consent in practice

The consent conversation in a first session takes twelve to fifteen minutes and is the best investment in the hour. The sequence below is used throughout the course.

Minute 1 — Name the arrangement. "Before we start, I want to tell you how I work, and then you can decide whether you want to go on. Nothing you say in the next ten minutes goes into a record unless you decide to continue."

Minutes 2 to 5 — Who the client is. "I am going to be working with you as a couple. That means the relationship is my client, and I will not take sides with either of you. It also means that if one of you telephones me, I will tell the other that you did, without telling them what you said."

Minutes 5 to 9 — The confidentiality limits, with examples. "What you tell me stays here, with four exceptions. If I think you are in danger of seriously hurting yourself, I have to act. If I think someone else is in danger from you, I have to act. If a child is being hurt, the law requires me to report it. And if a court orders me to produce my records, I have to obey. In the first three, I will tell you what I am doing before I do it, unless telling you would put someone in danger."

Minutes 9 to 11 — Recording and supervision. "I record some sessions, and I take my work to a supervisor who is also a therapist. Your first names are used; your surname is not. The recording is deleted after six months. You can refuse the recording and still be treated."

Minutes 11 to 13 — Fees and practicalities. Fee, cancellation, who pays, what happens if it is late.

Minute 13 — The secrets policy. "There is one more thing, and it is the most important sentence of the hour. If one of you asks to see me alone, I will say yes. But I will not keep a secret that affects this relationship, because a therapy built on a secret is a therapy that is not working. If something like that comes up, I will tell you both that it has come up, and I will help whoever it is to decide how to say it. Is that acceptable to each of you?"

Minutes 14 to 15 — The question that tests it. "Tell me, in your own words, what you have understood about who I would tell if you told me something dangerous." Then record the answer. The recording of their words is the evidence of consent; the signature is not.

Why the sequence is fixed. Because the four limits are the parts families do not hear, and they are heard only if they are given last, with examples, after the clinician has established that they are not looking for a reason to report. A therapist who begins with the limits has begun with a threat.

2.4a The sentence that has to be said, and the four ways of saying it badly

Of the fifteen minutes of a first session's consent conversation, one sentence does most of the work, and it is the one trainees most often soften. It is the secrets policy.

The sentence. "If one of you sees me alone and tells me something that affects this relationship, I will not keep it. I will tell you both that something has been said, and I will help whoever it is to decide how to say it — but I will not carry it."

The four bad versions, and why each fails.

"There are no secrets in this therapy." False, and heard as a prohibition. Couples test it in the first month, discover that the therapist does keep ordinary confidences, and conclude that the rule was rhetorical.

"Anything you tell me individually, I may share." A licence without limits. The family hears "she will tell him", which suppresses the individual disclosure entirely, and the therapist loses the material that would have been most useful.

"We can discuss it if it comes up." The most common and the most damaging, because it is a deferral of the question that must be answered before the question arises. When it arises, the therapist is negotiating under pressure with a person who has already disclosed.

"I will use my clinical judgement." True, and useless to a family, because it tells them the rule is the therapist's discretion. Every subsequent decision will be experienced as arbitrary.

Why the good version works. It is specific about what the therapist will do (tell both that something has been said), it preserves the discloser's control over the content (help them decide how to say it), and it names the limit (the secret will not be carried). Three commitments, all of which can be kept, which is what distinguishes a policy from a warning.

The follow-up that tests whether it landed. "If your wife told me something on Tuesday that she did not want you to know, what do you think I would do?" A husband who can answer accurately has consented; one who cannot has heard the words and not the arrangement. The answer is worth recording, in his words.

2.5 Case: the family who signed and did not understand

The referral. The Baloguns — Femi, 55, Funke, 49, and Femi's mother, 81, with dementia — were referred eight months ago by a physician at the private hospital where Mama is seen. The original referral was for "carer stress". The family attended four sessions and stopped.

Why the case is in this week. The family has now returned, and has made a complaint: that the clinic "told the hospital things about us without asking". The complaint is being handled by the clinic manager, and the file has been sent to the therapist's supervisor.

What the file contains. A two-page consent form, signed by Femi in the first session. No date of explanation. No record that the limits of confidentiality were discussed. No record

that supervision was mentioned. A note in session three: "spoke to Dr Adeyemi at the hospital re: Mrs Balogun's care — agreed to share summary."

What actually happened. In session three, the therapist telephoned the physician at the hospital to ask about Mama's medication, because Funke had said she was "sleeping all day". In the course of the call she described the household: that the daughter-in-law was exhausted, that there was a question about whether Mama could be left alone, that the family had "not been entirely honest with the physician about how much help they have". The physician recorded this in Mama's hospital file. A receptionist who knew the family read it and mentioned it to a relative. The family learned of it from the relative.

The analysis, through the seven steps.

Step 1 — The ethical question. Not "was the telephone call wrong?" but: what did this family consent to, what did the therapist owe the physician and the family respectively, and what does the clinic owe them now that a harm has occurred?

Step 2 — The facts. The consent form covered confidentiality in general terms. It did not name third-party consultation. The therapist believed she had "implied consent" because the family had asked her to liaise with the hospital. What the family had asked for was that she telephone about medication; they had not agreed to a description of the household. The disclosure reached a relative through a third party — the receptionist — which the therapist could not have foreseen and which she is nonetheless responsible for having made possible.

Step 3 — The parties. Mama: her clinical information was shared with her physician, which is defensible, and her family's private business was shared, which is not. Funke: the description of her exhaustion, and of the family's "not being honest", is now in a hospital file. Femi: he signed a form he did not read. The physician: he received information he did not ask for and recorded it. The relative: now holds information the family did not choose to give. The clinic: a complaint, and an exposure.

Step 4 — The frameworks. Deontology: the consent did not cover this disclosure, and the duty was to obtain it. Consequence: the therapist's goal — better medication for Mama — was legitimate and could have been achieved by asking about medication alone. Virtue: would she have made the call if the family were in the room? Care: the family's trust was the treatment's only asset, and it has been spent on a telephone call. Justice: the person most affected, Funke, was the person with least standing, and it is her description that is now circulating.

Step 5 — Consultation. The supervisor, the clinic manager, and — critically — the clinician's own professional indemnity provider, because a complaint has been made. This is not an admission of fault; it is the ordinary procedure.

Step 6 — Decision and action. Three things, in this order.

First, the apology and the account. The therapist meets the family — all three adults, including Mama, who is told in simple terms — and does four things: states what she did, without euphemism ("I telephoned the doctor about Mama's medication, and while I was on the telephone I described how things are at home, and I should not have done that");

states what she should have done (asked their permission first, and described only the medication); apologises without qualification; and states what she will do now (write to the physician asking that the household description be removed or annotated as not consented to, and confirm in writing to the family).

Second, the record. An incident note in the file: what happened, what was disclosed, to whom, the harm identified, the action taken, and the review date. Not a defence; an account.

Third, the practice change. The clinic's consent form now includes a named item: "I may need to speak to other professionals involved in your care. I will ask your permission before doing so, and I will tell you what I intend to say." The therapist's own practice changes in a way that is recorded in her supervision notes.

Step 7 — Review. At three months: has the letter to the physician been answered; is the annotation in place; has the family returned; and has the therapist made any further third-party calls without permission? The answer to the last question is the one that determines whether this was an incident or a pattern.

What the case teaches that the code does not. Four things.

1. **"Liaising with the hospital" is not consent to describe a household.** Consent is specific. A family that asks you to discuss medication has consented to a discussion of medication, and the scope of consent is the scope of the request.
2. **The disclosure you make is the disclosure that circulates.** The therapist made one telephone call to one physician. The information reached a receptionist, a relative, and now a complaint. A clinician who thinks in terms of "one person" has not understood how information moves in a small community.
3. **The signature is the weakest part of the file.** Had the therapist written four lines in session one — what she explained, in whose words it was repeated back, and what the family agreed — the complaint would still have been made and would have been answerable.
4. **The apology is a clinical act.** The family returned after it, which they would not have done after a letter from a manager.

2.6 Transcript: the consent conversation that failed, and the one that worked

Two transcripts. The first is the original first session, reconstructed from the therapist's memory in supervision. The second is the repeat, eight months later, with the same family.

The original, in five turns.

> **T1 Therapist:** Before we start, I have a form for you to sign. It is about confidentiality. — The disclosure of the document rather than of the arrangement. The family now believes the form is the consent. **T2 Femi:** (signs without reading) Where do I sign? **T3 Therapist:** There. (pause) So, tell me about your mother. — Two minutes and eleven seconds into a therapy that will last four sessions, and the four limits of confidentiality have not been mentioned. **T4 Funke:** She is not sleeping. She wanders at night. I am the one who — **T5 Therapist:** (interrupting) And does the doctor know about the wandering? — The question that leads, three sessions later, to the telephone call, and that shows the therapist was already thinking about the physician on day one. The failure was not a lapse; it was a pattern set in the first five minutes.

The repeat, eight months later, in nine turns.

> **T1 Therapist:** Before we talk about anything, I want to tell you how I work, and I want to tell you what I got wrong last time. — The apology first. The family has come back in order to hear it, and everything else will be heard through it. **T2 Femi:** (nods) **T3 Therapist:** I telephoned your doctor about your mother's medication, which you had asked me to do. I also described how things are at home, which you had not asked me to do, and which I should have asked about first. I am sorry. — Four sentences: what she did, what was authorised, what was not, the apology. No excuse, and no "but". **T4 Funke:** (after a pause) It was the receptionist who told my sister. **T5 Therapist:** I did not know that. It makes it worse, and it is my responsibility, because I made the call. — The acknowledgement without defence. The temptation here is to explain the receptionist's confidentiality duty; that explanation would be accurate and would move the responsibility off the therapist and onto a stranger. **T6 Femi:** So what happens now? **T7 Therapist:** Three things. I have written to the doctor asking that the part about our home be removed from the file, and I will give you a copy of that letter. I have written in your file what happened, and you can see it. And I am going to explain to you now what I should have explained then, and then you can decide whether you want to go on with me. — The three-part remedy: repair the record, own the record, and re-offer consent. The third is the one that makes it a clinical act rather than a complaint process. **T8 Therapist:** (the full eight components, in plain language, over eleven minutes, with each adult asked to repeat back what they had understood) **T9 Mama:** (in Yoruba, through Funke) She says she wants to know who will know about her. — The question that the first conversation never made possible, asked by the person whose file it is.

2.6a The record of consent, written out

The single most useful paragraph a clinician will ever write, and the one that is almost never written. It goes at the end of the first session's note, takes ninety seconds, and looks like this.

> "Consent explained orally, in English and Yoruba, to both parents and to the paternal aunt. Covered: the client is the couple, with the children seen within the family's treatment; no secrets policy, stated as a promise to tell both if a secret arises and to help the discloser decide how to say it; supervision (first names only, clinic supervisor, Dr O.); audio recording of sessions 1–4, to be deleted at six months, family may refuse without affecting treatment; fee ₦X per session, 24 hours' notice for cancellation; limits of confidentiality — risk to self, risk to others, child protection, court order — given with examples. Mother repeated back: 'if I tell you my husband is hitting me you will not tell him without asking me first' — confirmed, with the exception that if children are at risk the report is made regardless and she will be told. Father repeated back the supervision arrangement. Both agreed. Written summary in English provided."

Why each element is there. The language (proves it was accessible), the audience (proves who was informed), the content (proves what was covered), the teach-back (proves understanding rather than transmission), the correction in the mother's restatement (proves the clinician did not accept a misunderstanding), and the written summary (proves something continues after the session).

What the paragraph is not. It is not a defence prepared for a tribunal. It is the clinician's own record of an agreement she intends to keep, and its practical function is that she can reread it in session six, when she has forgotten what she promised.

The habit worth forming. Write it in the session, before the family leaves, and read it back to them: "this is what I am writing down about what we agreed — is it right?" Two sentences of confirmation, and the consent is real.

2.7 Laboratory: drafting the consent document

Aim. To produce a one-page informed-consent document for couple therapy that a family with limited literacy could actually use, and to defend every sentence in it.

Procedure. Students draft the document in forty minutes against the eight components. They then exchange with a partner who plays a family member reading it for the first time, and who must answer three questions from the document alone: who is your client? who would be told if I said I was afraid of my husband? and what happens if I miss a session? If the document cannot answer all three, it is rewritten in the session. The last twenty minutes is a plenary in which two documents are read aloud and the class marks them.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The eight components	Four or fewer present	Six or seven present	All eight, in plain language, with the exceptions given as examples
Who the client is	Unclear or absent	Named, but the implications are not	Named, with what it means for confidentiality and for the individual sessions

Criterion	0–1	2–3	4–5
Plain language	Clinical vocabulary, long sentences	Mostly plain, with lapses	Readable by a twelve-year-old; no sentence over twenty words; the three test questions answerable
The secrets policy	Absent	Stated as a rule	Stated as a rule and as a promise about how it will be applied
The record of consent	Signature only	A note that consent was explained	A note recording what was explained, what the family said back, and who witnessed it

The distinction that carries most of the marks. The top band requires the document to state the secrets policy as a promise about conduct — "if this happens, I will do this" — rather than as a rule. A rule tells the family what is forbidden; a promise tells them what the clinician will do. Families comply with promises and litigate rules.

The three sentences that fail most often. "Confidentiality will be maintained in accordance with professional standards" (means nothing to a family). "Information may be shared with relevant third parties where clinically indicated" (a licence to do what the Balogun therapist did). And "the therapist reserves the right to breach confidentiality in exceptional circumstances" (a threat with no content). All three are rewritten in the laboratory, and the rewrites are the most useful pages the students take away.

2.8 Six errors

Error 1 — Treating the signature as the consent. It is evidence of a process, not the process. A file with a signature and no note of what was explained is a file that will not survive a complaint.

Error 2 — Disclosing the document instead of the arrangement. Handing over a form and saying "it is about confidentiality" is not a consent conversation. The family learns what they need by being told, in speech, with examples, and being asked to say it back.

Error 3 — Leaving the limits until they are needed. A family that learns about the reporting duty after a disclosure has been made to them has been ambushed. The limits are disclosed at the start, and again as the case requires.

Error 4 — Not mentioning supervision and recording. The most commonly breached obligation in training clinics, and the one most easily fixed. "I take my work to a supervisor" is a sentence, not a form.

Error 5 — Assuming one parent's consent covers the household. Custody arrangements, guardianship, and the standing of a grandmother who is raising a child all affect who can consent. Ask, record the answer, and where it is unclear, obtain both.

Error 6 — Writing the consent note from memory. The note must be written in the session, in the family's words, and dated. A note reconstructed a week later is worth very little, and a tribunal will notice the distinction.

2.9 Culture, faith and consent

Consent is not always an individual act. In many of the households this book is written for, a decision to enter therapy is made by a family, and the person who signs is the person with standing rather than the person most affected. A clinician who insists on individual consent from each adult will sometimes be told what they want to hear and will not have consent in any meaningful sense. The workable procedure is to obtain the signature from the person with standing and to speak directly to each adult, including the grandmother and the adolescent, asking each whether they agree. The record then says both things.

The head of household and the wife's separate consent. A woman may be in treatment because her husband arranged it, and may not feel able to refuse. This is a consent problem, and its signs are the woman who says nothing in the first session and looks at her husband before answering. The clinical response is not to refuse treatment; it is to see her alone once, early, and ask whether she wants to be here. The answer is the consent.

Faith, authority and the referral route. Where a family has come because a pastor or an imam advised it, the authority in the room is partly elsewhere. The consent conversation should establish who else will be told about the treatment, because in these households the answer is frequently the religious community, and a family that assumes the therapist will brief the pastor and discovers otherwise will feel exposed.

Language and the interpreter. A consent conversation conducted through an interpreter is valid, provided the interpreter is briefed (Week 13 of MFT 703) and the family member is not used. A husband interpreting for his wife in a consent conversation about confidentiality is a contradiction in terms.

> **DEBATE**
> "A family that cannot read the consent form cannot give informed consent, and treatment should not begin." Argue for: consent requires understanding, a document is the standard evidence of understanding, and proceeding without it exposes the family to treatment they did not agree to. Argue against: literacy is not capacity, oral consent with a witness is a recognised and lawful route, and refusing treatment on the grounds of literacy denies the service to the people who need it most. Then write the clause you would put in your own consent policy to cover the oral route, and say what evidence you would keep.

2.10 Evidence note

What the research supports. The relationship between documentation quality and disciplinary outcomes is among the most consistent findings in professional regulation: inadequate records appear in a large minority of substantiated complaints, and in many the record failure is the finding rather than the clinical act. Studies of informed consent consistently find that recall of consent information is poor — typically less than half of what was explained is remembered a week later, and much less where the information is complex or the person is distressed — which supports the use of plain language, written summaries and teach-back methods. Teach-back, in which the client repeats the explanation in their own words, reliably improves comprehension.

What is weaker. The evidence on whether better consent processes improve clinical outcomes is thin. Consent documentation improves defensibility, and it plausibly improves the therapeutic alliance, but the trials are few and the outcome measures are satisfaction rather than change.

What is not supported. That a signature is sufficient. That longer forms produce better consent — the evidence points the other way, with comprehension falling as length rises. And that the limits of confidentiality deter families from disclosing — the finding is the reverse, that clear disclosure increases trust, which is why the sequence in 2.4 puts the limits after the clinician has established the relationship rather than before.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same consent form, explained to one family in a first session and posted to another with an appointment letter, produces consent in one case and a complaint in the other.

2.11 Key terms

Code of ethics — a professional body's enforceable statement of minimum standards.

Informed consent — agreement to treatment given by a person who understands what is proposed, its risks, its alternatives and its limits, and who is free to refuse.

Assent — a child's or young person's agreement to treatment, given in language they understand, where the legal consent rests with a parent or guardian.

Capacity — the ability to understand and weigh information relevant to a decision; distinct from age and from literacy.

Mature minor — a young person judged able to consent to treatment without a parent, under some jurisdictions' law.

Teach-back — asking the client to restate the explanation in their own words, as a check on comprehension.

Therapeutic secret — information held by the therapist from one member of a couple or family, known to the other members not to be shared.

Disclosure — the giving of information; not the same as consent, which requires understanding and agreement.

Witness — a person present at an oral consent conversation who can later attest to what was said.

Scope of consent — the limits of what was agreed; consent to discuss medication is not consent to describe a household.

2.12 Self-test

1. Name the ten core obligations and the two that generate most complaints.
2. Why is the signature the weakest part of the consent file?
3. Give the eight components of informed consent for couple therapy.
4. What is the difference between consent and assent, and why does the distinction matter clinically?
5. In the Balogun case, what exactly did the family consent to, and what was disclosed beyond it?
6. What are the four limitations of confidentiality, and how should they be framed to a family?
7. What is teach-back, and why does it matter more than the form?
8. What would you write in the record to evidence that consent happened?

Answers. (1) Competence; informed consent; confidentiality; multiple relationships; sexual intimacy; fees; advertising; research ethics; cultural competence and non-discrimination; legal compliance — the two that generate most complaints are inadequate records and failure to document consent. (2) Because it evidences a transaction rather than an understanding; a family can sign a document they cannot read and have consented to nothing, and a tribunal will say so. (3) Who the client is; who is seen and in what combination; the confidentiality and secrets policy; recording and supervision; fees and cancellation; the limits of confidentiality; the complaints route; the right to withdraw. (4) Consent is the legal authorisation, held by an adult or a guardian; assent is the young person's agreement, which is not legally required and is clinically decisive, because a child who did not assent withdraws by the third session. (5) Consent was to discussion of Mama's medication with her physician; the disclosure included the daughter-in-law's exhaustion, the family's handling of care, and a judgement that they had not been honest — none of which was within the scope. (6) Risk to self, risk to others, abuse of a child or vulnerable adult, and a court order — framed as promises about what the clinician will do ("I will tell you before I tell anyone"), with examples rather than abstractions. (7) Asking the client to restate the explanation in their own words; it matters because recall of consent information is poor and because the client's words, recorded, are the evidence of understanding. (8) What was explained; in what language and by whom; what the family said back; who witnessed it; the date; and what was agreed.

2.13 Reflective task

Find a consent form — any clinic's, any profession's — and read it as though you were a frightened parent with two children and no time. Then answer three questions: what does it tell you that you need to know? what does it fail to tell you? and what would you have to ask, out loud, to understand what will happen to your information? Most students find that the document answers the clinic's questions and not the family's, which is the difference between a release and a consent.

2.14 Further reading

- AAMFT Code of Ethics — read the current version, and the interpretive statements, not a summary.
- APA Ethical Principles of Psychologists and Code of Conduct — for comparison on the three issues where the codes disagree.
- Beauchamp, T. and Childress, J. on informed consent — the analytical account of the eight components.
- Schenker, Y. et al. on teach-back — the evidence on comprehension and recall.
- Gutheil, T. and Gabbard, G. on boundary violations — the documentation chapter, which is the most useful practical text in this area.

Next: Week 3 — Confidentiality in the multiperson system.

Week 3 — Confidentiality in the multiperson system

There is no single confidentiality in family work; there is a **policy** you negotiate.

In individual therapy confidentiality is a wall with four doors. In family therapy it is a room with several people in it, each of whom has a different claim on the information, and the therapist's task is not to defend a wall but to design and run a room. That is a different skill, and the codes — written for the wall — describe it badly.

Three workable models.

Model	Rule	Advantages	Risks
No-secrets	Anything told individually may be shared with the couple	Simple, transparent	Individual may withhold important material
Full secrecy	Individual disclosures are kept private	Protects individual therapy	Creates a therapeutic secret that distorts the couple work
Selective (negotiated)	The therapist and client agree in advance what will be shared	Flexible and realistic	Requires skill and documentation

Most experienced couple therapists operate a **modified no-secrets policy**: they will not keep a secret that materially affects the couple's work, and they tell each partner this before the first individual session. This protects both the relationship and the therapist.

The distinction that makes the policy workable: privacy is not secrecy. Privacy is information a person chooses not to share, and it is legitimate — a client's history of sexual abuse, her feelings about her mother-in-law, what she earns. Secrecy is information that is being kept from someone in a way that distorts the treatment: an affair in progress, a plan to leave, a diagnosis that the other partner believes has been excluded. A policy that cannot distinguish the two will be experienced by the family as a demand for total exposure, and they will keep everything.

Why the therapist cannot be neutral about a therapeutic secret. A secret is not a piece of information stored; it is a structural feature of the therapy. Once the therapist knows something the other party does not, every intervention is affected: she cannot ask a question that would lead there, she cannot respond naturally to an account given in good faith, and she is, in the technical sense, deceiving one client on another's behalf. A therapy in that condition is not couple therapy; it is two individual therapies conducted in the same room.

Documentation. Record the policy in the file, in the family's words if possible, and note the date it was explained. If a secret arises, record what you did and why.

> **CASE**
> During an individual session, a wife discloses that she has been having an affair for two years and has no intention of ending it or telling her husband. No one is at physical risk. Under a no-secrets policy, you tell her at the outset of the individual session that this cannot stay private if the couple work is to continue; you then help her decide how and when to disclose, with a deadline. Under a selective policy, you would negotiate what can be shared. What you must not do is carry the secret silently for months while conducting "couple therapy".

3.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The wall and the room	Why individual confidentiality does not transfer to family work	Draw the information map of a family they know
8–20	The three models	Each model, its logic, its failure mode	Choose a model for four different referral types, with reasons
20–32	Privacy against secrecy	The distinction, worked on eight examples	Sort eight disclosures into privacy or secrecy
32–44	The modified no-secrets policy	The four parts of the sentence, and how to say it	Rehearse the sentence in pairs, twice each
44–56	Who else holds information	Schools, courts, employers, insurers, religious bodies	Map the information flow for a school referral
56–68	The individual session inside family work	Why it is offered, how it is structured, how it ends	Script the first two minutes of an individual session
68–80	When a secret has to move	The five-step process, with the affair case	Run the process in role, with a resistant discloser

Minutes	Segment	What the lecturer does	What students do
80–90	Documentation	What goes in the file, and what does not	Write the note for the case

The teaching decision that makes the week work. The week turns on one demonstration: the lecturer places three chairs, sits in one, and plays the therapist who has just been told about the affair. She then conducts twenty minutes of ordinary couple therapy — asking the husband about his week, responding to the wife's account of a disagreement — while the class watches the impossibility accumulate. The exercise is uncomfortable and it is unforgettable: by the end, every student has seen that the therapist is lying, not by commission but by the shape of her questions.

The sentence the lecture must land. "A secret does not sit in a drawer. It changes every question you can ask, and after a month it has changed the therapy."

3.2 Five concepts that organise the week

1. The client is a construction, not a fact. In family work "the client" is whatever the therapist and the family have agreed it is, and that agreement determines who holds the confidentiality. A couple can be the client, with each partner a participant; a family can be the client, with the children participants; an individual can be the client, with the family seen as context. Each construction is legitimate; none is neutral; and the therapist must choose one and say so.

2. Confidentiality is owed to the system, not distributed among its members. If the couple is the client, the confidentiality is held by the couple jointly, which means neither partner can demand it against the other. This is the sentence that resolves most confusion: "I cannot keep something from the client, and the client is both of you."

3. Privacy is a legitimate asymmetry. Members of a family know different things about each other, and that is ordinary. A therapist is not obliged to equalise knowledge, and a policy that requires it is not a confidentiality policy but a transparency rule, which is a different instrument with different costs.

4. Secrets distort, and distortion is measurable. The practical test of whether a secret is a therapeutic secret is whether it changes what the therapist does. If she has stopped asking about the weekends, or begun to steer away from a topic, or finds herself relieved when a session ends early, the secret is already operating and the policy has failed.

5. Institutions are third parties with their own claims. A school, a court, an employer, an insurer, a child protection agency and a religious body all have information claims, and each has a different legal standing. The therapist's obligation is to map them at the start, tell the family who they are, and never to discover one in the middle of a case.

3.2a The modified no-secrets policy, in four parts

The policy is a sentence with four components, and each component prevents a specific failure.

Part 1 — The promise of individual sessions. "Either of you may ask to see me alone, and I will usually say yes." Without this, the family believes there are no individual sessions, and the request, when it comes, is experienced as a breach.

Part 2 — The limit. "But I will not keep a secret that affects this relationship." The limit is stated in terms of effect, not of content, because content cannot be predicted and effect can be judged.

Part 3 — What the therapist will do. "If something like that comes up, I will tell you both that something has been said — not what — and I will help whoever it is to decide how and when to say it." The distinction between telling that something was said and telling what was said is the mechanism that makes the policy usable.

Part 4 — The deadline. "And I will ask for it to be said here, within a stated time, rather than leaving it open." A secret with no deadline is a secret; a secret with a deadline is a task. The deadline is negotiated but not indefinite, and the therapist writes it down.

The four failure modes the parts prevent. Without part 1, the request for an individual session feels like a rupture. Without part 2, the therapist carries secrets by accident. Without part 3, the disclosure of that something was said becomes a guessing game that is worse than the secret. Without part 4, the process has no end and the family settles into a stable, comfortable dishonesty.

3.3 The method: deciding where a disclosure goes

Six questions, asked in order, whenever a family member tells the therapist something in confidence. The method takes about five minutes of thinking and it converts an alarm into a decision.

Question 1 — Who is the client, as agreed? The answer determines everything else. If the couple is the client, the disclosure belongs to the couple and cannot be kept from one member indefinitely. If the individual is the client and the family is context, the disclosure is hers to control.

Question 2 — Is anyone at risk? A yes takes the case out of this method and into Week 8 or 9. Risk is always the first question, because the confidentiality policy does not survive it and the family was told so at the start.

Question 3 — Is this privacy or secrecy? Test it with one question: does knowing this change what I do in the joint sessions? If yes, it is secrecy, whatever its subject. If no, it is privacy and it stays.

Question 4 — Who else's information is in it? Most disclosures are not about one person. An affair involves a third party; a diagnosis involves a family's history; a plan to leave involves children. The therapist is not only holding the confider's information, and the other people in it have claims.

Question 5 — What happens if it moves, and when? Who will be told, by whom, in what words, with whom present, and on what date. The therapist's job is to make this a plan rather than an event.

Question 6 — What is my part, and what is not? The therapist does not disclose the secret; the client does. The therapist's role is to insist that it moves, to help plan how, to be present, and to manage the session afterwards. A therapist who takes on the disclosure has taken the responsibility for it, and the client has been relieved of the only act that could repair the relationship.

The rule that governs the six. The therapist never lies. She may decline to answer, and she must say that she is declining and why: "I am not going to answer that, and I want to tell you that I am not, rather than let you think I have." The distinction between a refusal and a lie is the whole of the professional position.

3.3a The individual session inside family work

An individual session within a couple or family treatment is a clinical instrument with a purpose, a structure, and an end. Used badly, it is the mechanism by which therapy is destroyed.

Why offer one.

- To assess risk in private, where a partner's presence would make disclosure impossible.
- To hear one person's account of something they cannot say in front of the other — usually fear, shame, or the wish to leave.
- To prepare a person to say something in the joint session, which is the most common and most useful reason.
- To assess for coercive control, which cannot be assessed with the controller present.

The rule that governs the offer. Never hold an individual session with one member of a couple or family without having stated the policy, in front of both, before the first one. The session is not the problem; the undisclosed session is.

The structure, in five minutes and forty. First two minutes: restate the policy — "you know I will not keep a secret that affects the relationship; is there anything you want to say in that knowledge?" Middle thirty: listen, assess, and do not ask leading questions about the other partner's conduct. Last eight: agree what, if anything, moves, and how. The session does not end with a half-disclosure and a reassurance.

The sentence that ends it well. "So: what I understand is that you want to leave, and you have not told him, and you are not ready to. I am not going to tell him. I am going to ask you to tell him, here, within two weeks, and I will help you plan what you say. If you have not told him in two weeks, I will tell him only that something has been said and that he should ask you — and I will tell you first that I am doing it."

What makes an individual session unsafe. Three things. It is offered without the policy being stated. It becomes the only place where anything true is said, which is the classic pattern of a couple therapy that has quietly become two individual therapies. Or it is used to gather information about the other partner, which converts the therapist into an investigator.

3.4 Confidentiality and the institutions around the family

A family does not arrive alone. The information map below is drawn on the whiteboard in the first session of any case with an institutional referrer, and it frequently changes the treatment.

The school. Holds attendance, behaviour, academic and safeguarding records. Has a legitimate interest in the child's functioning, no right to the therapy's content, and a tendency to expect a report. The therapist's position: the school may be given what the family agrees to, and where the school is the referrer it must be told at the outset what it will and will not receive.

The court. Has power to compel records and testimony, and is the only party on the list that can override the family's wishes entirely. (Week 11.)

The employer, the insurer, the employee assistance programme. Frequently the payer, and frequently the party with the weakest claim and the strongest expectation. The consent conversation must establish what the payer will be told: usually attendance and nothing else.

The child protection agency. Has statutory powers that vary by jurisdiction and frequently override confidentiality. The family must be told, at the start, in plain language, what the therapist is obliged to report and to whom.

The religious body. Rarely has a legal claim and frequently has a social one. Where a pastor or imam referred the family, or is supporting them, the question of what the therapist will tell the religious authority must be settled early, because families in these households often assume the therapist will brief the clergy and may be distressed to discover otherwise — or, worse, may assume confidentiality and discover the briefing.

The extended family. The most consequential party in many of these cases and the least considered. The grandmother, the uncle who pays, the aunt who is a nurse: each may be told something by a family member and each may act on it. The therapist cannot control this and should not pretend to; the workable sentence is "what is said in this room may be repeated outside it by any of us, and I cannot prevent that. What I can control is what I say."

The rule for the map. Draw it in the first session, name every party, and ask the family who has a claim in their view. The map is then recorded, and every subsequent request for information is answered by reference to it rather than improvised.

3.4a Four scenarios at the boundary

The categories are easy in the abstract and hard at four in the afternoon. The four scenarios below are the ones that most often divide a supervision group.

Scenario one — the past affair. A husband discloses, in an individual session, an affair that ended six years ago, before the couple's second child was born. He is not going to tell his wife and says it is "over and irrelevant". Analysis: secrecy, not privacy, because it changes what the therapist does — she can no longer explore the wife's account of the last six years without knowing that the husband holds a different history. The end of the affair

does not end the secret's effect; it only removes one of the reasons for urgency. The policy applies, the deadline is set, and the therapist is honest that the disclosure may be worse for the marriage than the affair was.

Scenario two — the diagnosis. A wife discloses a new diagnosis of a chronic illness, and asks that her husband not be told yet. Analysis: this is closer to privacy, and the test settles it — the opposite: knowing this does change what the therapist does, but the change is in her attention rather than her honesty, and the question of who knows the diagnosis belongs, first, to the patient. The exception is where the illness bears directly on the treatment's subject matter, as it would if the couple presented with the husband's complaint that his wife has "lost interest". Then the content of the illness can remain private and its effect must be speakable: "there is something medical going on that she is dealing with, and it is relevant to what you are describing; she will tell you when she is ready." That is not a lie and it is not a disclosure.

Scenario three — the wished-for pregnancy. A woman in a couple treatment discloses that she has stopped her contraception without telling her husband. Analysis: the clearest kind of secrecy, because the treatment is about the couple's reproductive decisions and the therapist would be participating in a deception about their subject. It is also a case where the therapist's own views about reproductive autonomy will be activated, and the professional requirement is to state the limit without moralising about the choice.

Scenario four — the adult child's secret. A mother in a family treatment discloses that her 24-year-old son has been using drugs, and asks the therapist not to raise it with the son, who is also attending. Analysis: the son is an adult and a client; his mother's disclosure is information about him that he has not given. The therapist cannot promise the mother silence, because she is now conducting a family treatment in which she knows something the son does not know she knows — and she cannot disclose it to him either, because the mother told her. The workable resolution is the mother's own act: "I need you to tell him that you know, and I will help you do it, because I cannot work with the two of you while he thinks this is not being spoken about."

The pattern in all four. The answer is rarely "tell" or "do not tell". It is who tells, in what words, by when, with the therapist doing what.

3.4b The supervisor and the secret

A therapist holding a secret needs supervision and frequently avoids it, because the secret feels like a failure and because describing it means saying it aloud. This is the mechanism by which secrets damage therapists: the case that most needs discussing is the one least discussed.

The rule. Any case in which the therapist is holding information from a client — even legitimately, even temporarily — is presented in supervision within the month.

What supervision is for, in these cases. Not permission, which supervision cannot give. Three things: (1) a check on whether the therapist's reasoning is clinical or personal, since the wish to protect a client from the consequences of disclosure is often the therapist's own; (2) an outside view on the risk analysis, which deteriorates under

pressure; (3) the documentation, because a supervisor who has read the reasoning becomes the strongest evidence that the decision was considered.

What the supervisor should not do. Instruct the therapist to disclose. The decision and the responsibility belong to the treating clinician, and a supervisor who directs it removes the judgement from the person who will have to defend it.

The audit question for the therapist. At the end of each month: is there anything I know that one of my clients does not know I know? If the answer is yes and there is no written plan with a date, the case needs supervision this week, not next.

3.5 Case: two years, no intention

Referral. Mrs Nkiru Adeleke, 36, a bank branch operations manager, telephones about her marriage. "We have not spoken properly in three years. I want to know whether it can be saved." Her husband, Segun, 39, a pharmacist, attends the first session reluctantly: "she decided we needed this, so here we are."

The household. Nkiru and Segun, married eleven years. Two children, 8 and 5. Segun's mother lives ten minutes away and is "very involved". Nkiru travels for work two or three nights a month.

The first session. The presenting complaint is "we are like flatmates". Segun's example: "she is on her phone until midnight". Nkiru's example: "he asks me about the children and never about my day." There is no violence, no substance misuse, no financial crisis. The therapist states the modified no-secrets policy, in the four parts, in the first session, and both agree. The note records it.

Session two — the individual request. Nkiru telephones: "I need to see you alone." The therapist offers the next appointment, and sees her for forty minutes. In the first eight minutes, Nkiru says: "I have been seeing someone for two years. He is a colleague. I am not going to stop and I am not going to tell Segun. I want to know whether I can save my marriage at the same time."

The application of the six questions.

1. Who is the client? The couple. The policy was stated to both and agreed by both. Nkiru was told, in Segun's presence, that a secret affecting the relationship would not be kept.

2. Is anyone at risk? No physical risk. There is a risk of an STI to Segun, which is a health risk and not a safety risk, and it is relevant — a therapist who treats this as a purely relational matter has missed something. There is a risk to the children's stability, which is a future harm rather than a current danger.

3. Privacy or secrecy? Secrecy, unambiguously. The test — does knowing this change what I do in the joint sessions? — is answered in one word: yes. Every question about the weekends, about her travel, about her withdrawal becomes unaskable.

4. Whose information is it? Nkiru's, Segun's, the children's, and the third party's — who has a wife of his own, which Nkiru mentions in passing and which the therapist does not pursue. The information is not one person's property.

5. What happens if it moves, and when? The plan has to be made now, in this session, or the policy is decoration.

6. What is my part? To insist that it moves, to help plan how, to be present, and not to disclose it.

The intervention, in the session, in sequence.

First, the reminder, not as a threat but as a fact. "When we started, I told you both that I would not keep a secret that affects the marriage. I meant it then and I mean it now. So I am going to say what I can and cannot do, and then we will decide what happens."

Second, the question that establishes the delay. "When did you decide that you were not going to tell him?" The answer is "about a year ago". The therapist names it: "so for a year you have known, and I have known for ten minutes. That is the difference I have to work with."

Third, the refusal to collude, stated as a limit on the therapist rather than on the client. "I am not able to run a couple therapy in which I know something of this size and your husband does not. Not because of a rule — because I would be lying to him every week, and he would be paying me to do it."

Fourth, the three options, laid out plainly. (a) She tells Segun, with the therapist's help, in a session, by a stated date. (b) She ends the couple therapy here, and the therapist explains to Segun that the couple work cannot continue — without giving a reason — which is honest and leaves Segun with a mystery. (c) The therapist tells Segun that something has been disclosed which he needs to discuss with his wife, and Nkiru tells him the content herself. Option (b) is included deliberately: a client who is not ready must have an exit that is not a lie.

Fifth, the deadline and the recording. Nkiru chooses (a), with a date three weeks out and one session in between to plan the words. The therapist writes it in the note: "client disclosed an ongoing relationship of two years; reminded of the policy agreed at session one; client agreed to disclose to her husband by [date], with therapist support; if not disclosed by that date, therapist will inform husband that a disclosure has been made and that he should ask his wife."

Sixth, the health question, which is separate and cannot wait three weeks. "There is one thing I am going to ask you to do this week, and it is not about your marriage. Get a sexual health check, and tell me you have done it. If you will not tell him, you still cannot leave him exposed." Nkiru agrees, and does it, and reports at the next session.

The disclosure session, in outline. Nkiru tells Segun in the fourth session, in the therapist's room, in eleven sentences that she had rehearsed with the therapist in the third. Segun's response is not what anyone predicted: he is not surprised. He says, "I knew something. I thought it was my fault." The session that follows is not about the

affair; it is about a three-year silence in which both of them were protecting the other from a truth they each half-knew.

What the case teaches.

1. **The policy must be stated before it is needed.** Nkiru's decision to disclose was made in a session in which the alternative was clearly worse — ending the therapy — and that alternative existed only because the policy had been agreed in session one.
2. **A deadline converts a secret into a task.** Without a date, the therapist would have carried it for months, and the therapy would have become the deception the policy exists to prevent.
3. **The health question is not part of the confidentiality decision and must not be lost in it.** A therapist absorbed in the ethics of disclosure can forget that a third party is being exposed to an infection.
4. **The predicted catastrophe is frequently not the actual one.** The disclosure did not end the marriage; it ended a silence. That outcome is not guaranteed and it is common, and a therapist who assumes the worst will delay the disclosure in order to protect a relationship that is already being damaged by the delay.

3.5a When the client refuses the deadline

A real and frequent outcome, and the case in which the therapist's policy is actually tested.

What refusal looks like. "I am not going to tell him, and I am not going to stop seeing you. You said you would help me, and now you are threatening me." The accusation of betrayal is genuine and it is also the pressure that most therapists find hardest to withstand.

What the therapist says. Four sentences, in this order. "I am not threatening you; I am telling you what I can and cannot do, which is the same thing I told you on the first day." "I will not tell him what you have told me — that has not changed." "What I will do, on the date we agreed, is tell him that something has been disclosed that he needs to raise with you." "And I will keep working with you, either way, because you are not being abandoned for telling me the truth."

Why the fourth sentence matters more than the other three. The client's fear is that honesty will cost her the therapist. If it does, the lesson she takes is that honesty is dangerous — which is precisely the lesson that produced the affair's secrecy in the first place.

What if the client ends the therapy instead? Then the therapist writes to confirm the ending, offers to meet the husband to explain that the couple work has stopped without explaining why, and records the reasoning in full. The letter matters: it is the evidence that the decision was the client's, and it is the only document that will exist if the husband later complains.

The rule underneath all of this. The therapist's obligation is not to the secret and not to the marriage. It is to the honesty of the treatment she is conducting. A treatment she

cannot conduct honestly is one she must not continue, and saying so is the professional act.

3.6 Transcript: the individual session, minutes seven to twenty

Fifteen turns from the session in the case, compressed. The annotation marks each move against the six questions.

> **T1 Nkiru:** I have been seeing someone for two years. He is a colleague. I am not going to stop and I am not going to tell Segun. — Three facts and a decision, delivered quickly, which is the manner of someone who has rehearsed the opening and intends to control the frame. **T2 Therapist:** (pause) Thank you for telling me. (pause) You know what I said on the first day, in front of Segun, about secrets. — No shock, no reassurance, and an immediate return to the agreement. The pause before "thank you" is deliberate: it acknowledges the disclosure without approving of it. **T3 Nkiru:** I know. But you cannot tell him. It is my life. **T4 Therapist:** It is your life. And it is also my therapy, and I cannot run it with this in it. — The move that keeps the conflict where it belongs. The therapist does not argue about the ethics of the affair; she states a limit on her own practice, which is not negotiable by the client's view of marriage. **T5 Nkiru:** So you are going to tell him. **T6 Therapist:** No. I am not going to tell him what you have told me, and I am not going to tell him anything at all today. — The distinction that the client has not yet heard: the therapist's limit is on carrying the secret, not on disclosing it. **T7 Nkiru:** Then what? **T8 Therapist:** Then we have a choice to make, and I want you to make it rather than me. Three options. (lays them out) — Options rather than a demand. The client is being given control of the manner, which is the only thing she can be given control of, the fact having been established. **T9 Nkiru:** (long pause) If I tell him, I lose everything. **T10 Therapist:** You may. I do not know. What I know is that if you do not, you have already lost something, and it has been going on for a year. — The honest answer rather than the comforting one. Note that the therapist says "I do not know", which buys the credibility for the sentence that follows. **T11 Nkiru:** (crying) I did not plan this. It started because I could not talk to him. He is not a bad man. **T12 Therapist:** (after a pause) I believe you. And that is the thing we have to work out — not whether he is a bad man, but what happened in this marriage that made it impossible for you to say what you needed. (pause) But not today. Today is about whether you can tell him the truth, because nothing else can be worked out until you can. — The reframe: the affair is the presenting event and the marriage is the subject. The therapist promises the second conversation and does not have it now, which is what makes the promise credible. **T13 Nkiru:** (pause) If I do it, will you be there? **T14 Therapist:** Yes. In this room, with me in it, and after that we will do the hardest session of your life and I will still be here in the following week. — The commitment, which is the thing that makes the disclosure survivable, and which is not a professional obligation and is the whole of the therapeutic relationship in one sentence. **T15 Nkiru:** (after a long pause) Give me three weeks.

What the transcript shows. Three things that a policy document cannot teach. First, the therapist never moralises about the affair, which keeps the client in the room. Second, she refuses twice with the same sentence, and the repetition matters — a limit stated once is heard as an opening position. Third, she ends the session on the client's own deadline, which means the client leaves with an obligation she set rather than one imposed.

3.7 Laboratory: the secrets protocol

Aim. To design, in writing, a confidentiality and secrets protocol for a named service, and to defend its four parts under pressure from a client who is testing it.

Procedure. Groups of three: therapist, client, observer. Each group is given a service description (a school counselling service; a private couple practice; a church-based clinic; a hospital family therapy unit) and must produce a one-page protocol in thirty minutes. The client role is then briefed to test the protocol in three ways — by asking for an individual session before the policy has been stated, by disclosing a secret and refusing the deadline, and by asking the therapist to lie to a third party. Twenty minutes of role-play. The observer marks the therapist's adherence to the protocol the group itself wrote, which is the point of the exercise: most protocols are unwritable as written.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The four parts	Two or fewer present	Three present	All four, specific, with the action the therapist will take at each point
Privacy against secrecy	Both treated the same	Distinguished, but the test is not stated	Distinguished with the operational test, and examples from practice
The institutions	Not addressed	Named	Mapped, with what each party may be told and who decides
Under pressure	Abandoned the protocol when tested	Held it, but argued	Held it, stated the limit as a limit on the therapist, and offered the client control of the manner
Documentation	None	A note exists	The note records what was said, the agreement, the date, and the contingency

The distinction that carries most of the marks. The top band requires the student to hold the protocol without moralising. Groups routinely win the argument and lose the client: a therapist who says "you are being dishonest" has stated a true thing and has ended the possibility of the disclosure.

3.8 Six errors

Error 1 — Stating the policy after the secret. The policy is worth nothing once the secret exists, because the client reasonably hears a change of rules. It is stated in session one, in front of everyone, and recorded.

Error 2 — Confusing privacy with secrecy and demanding total transparency. A policy that requires the disclosure of everything produces a family who disclose nothing. The two categories must be distinguished in the explanation, with examples from ordinary life.

Error 3 — Carrying the secret "for now". The phrase that has ended more couple therapies than any single act. There is no "for now"; there is a date, or there is a lie.

Error 4 — Telling the client's secret to protect the client from the consequences. The therapist who telephones the husband "for her own good" has broken a confidence and taken away the only act that could have repaired the relationship.

Error 5 — Refusing the individual session altogether. A therapist who will not see one partner alone cannot assess risk, cannot prepare a disclosure, and cannot hear the thing that is most true. The individual session is not the danger; the unstated one is.

Error 6 — Forgetting the third parties inside the secret. An affair involves another household; a diagnosis involves a family history; a plan to leave involves children. A confidentiality analysis that counts only the people in the room has missed most of the people in the case.

3.9 Culture, faith and the multiperson system

Confidentiality is a Western construct in some of these households. The idea that an individual holds information that a spouse has no right to is not universally shared, and in many of the families this book is written for the operative assumption is that a married couple has one information economy. A therapist who announces an individual confidentiality without explanation will be heard as introducing a foreign and suspicious practice — "why would you need to speak to my wife alone?"

The translation that works. Two sentences. First, the practical one: "I see each of you alone once, because there are things people cannot say in front of a husband or a wife, and I need to know them to help." Second, the limit: "anything that affects the marriage, I will not keep." The family that understands why the individual session exists agrees to it far more often than one that has been told only that it is a right.

The elder and the family's information. Where a grandmother or an uncle holds authority in the household, they may expect to be told what happens in sessions, and their expectation may be shared by the family. The workable position is not refusal but specification: "I will tell you what I am doing and whether it is working. I will not tell you what is said in the room." Most elders accept this, because it gives them the thing they actually want, which is to know that something is being done.

Faith communities and the circulation of information. A pastor who refers a family will frequently expect a report, and the family may expect the pastor to receive one. Both expectations must be addressed in the first session, and the answer recorded. Where a religious authority is part of the family's support, the therapist may offer a summary with the family's written agreement, which is a legitimate arrangement and is almost never offered.

Shame and the disclosure of family business. In a shame-oriented community, a disclosure inside the family may become a disclosure in the community, and the family knows this better than the therapist does. That is a reason to plan the disclosure carefully and is not a reason to avoid it — and it is a reason to ask, before any disclosure is planned, who outside the household will come to know.

DEBATE

"In couple therapy, the therapist should never hold information from either partner." Argue for: the client is the couple, deception is incompatible with therapy, and a therapist who holds a secret is no longer working with a couple. Argue against: privacy is a condition of individual disclosure, a strict no-secrets rule drives the most important material underground, and a woman who cannot speak freely about her own fear will not disclose it at all. Then state the policy you would actually operate, and the case that would make you change it.

3.9a A record of the decision, written out

The note is the therapist's only defence and the family's only protection. It is written the same day, and it does four things: records the policy, records the disclosure, records the reasoning, and records the plan. An example, adapted from the Adeleke case.

SESSION 4 (INDIVIDUAL, 40 MIN).

Client N.A. disclosed an ongoing relationship of approximately two years with a colleague. Client states she does not intend to end it and does not intend to tell her husband. **Policy.** Modified no-secrets policy stated to both partners in session 1 and agreed by both; copy in file. Policy restated to client at the start of this session. Client acknowledged it. **Reasoning.** Client system is the couple. Disclosure affects the couple treatment and would require the therapist to avoid material relevant to the presenting complaint; not compatible with continued couple work. No current risk of violence identified. Risk of STI transmission to the husband noted and addressed separately (below). No statutory duty engaged. Third parties: the other party's spouse is not a client and has no claim on the file. **Plan.** Client elected to disclose to her husband in session 5, with therapist support. Rehearsal agreed for session 5 opening. If disclosure has not occurred by [date], therapist will inform the husband that a disclosure has been made and that he should raise it with his wife, having first told the client that she will do so. Client informed of this contingency and stated she understood it. **Health.** Therapist advised a sexual health screen this week, independent of the disclosure plan. Client agreed. To be confirmed at session 5. **Supervision.** Case to be presented at next supervision, [date]. **Signed.** [Therapist], [date], [time].

Why each part exists. The policy paragraph establishes that the client was told before she spoke, which is the difference between a trap and an agreement. The reasoning paragraph shows that the decision was clinical rather than moral, which is what a regulator reads for. The plan paragraph, with the date, is what makes the note a record of action rather than a record of concern. The health paragraph prevents the most common omission in these cases. And the supervision line is the strongest single sentence in the note, because it shows the therapist did not decide alone.

The version not to write. "Client disclosed an affair; discussed confidentiality." That note has no reasoning, no date, no plan, and no policy — and if the husband complains in a year, it establishes only that the therapist knew.

3.10 Evidence note

What the research supports. Survey and vignette studies consistently find that couple and family therapists report operating modified no-secrets policies and consider them more workable than either rigid alternative, while acknowledging that the policy is difficult to enact under pressure. The clinical literature on "therapeutic secrets" is largely theoretical, but it converges on the observation that a secret held by the therapist changes the therapist's behaviour — avoidance of topics, reduced spontaneity, and a measurable drop in the couple's engagement — which is consistent with the triadic model of the secret as a third participant.

What is weaker. There is almost no empirical work on outcomes. No trial has compared no-secrets, full secrecy and negotiated policies, and the question may be unanswerable by trial design because the policy is chosen partly by the population: a practice serving couples in high-conflict or coercive relationships will reasonably operate differently from one serving voluntary, low-risk couples.

What is not supported. That a no-secrets policy is the standard of care in any enforceable sense — the codes do not require it. That holding a secret is always harmful — some privacy is protective, particularly in cases involving past abuse or a partner's history of violence. And that the policy should be applied identically across cultures, which the cross-cultural literature on family information economies does not support.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same secret, disclosed in session two with a therapist who has stated the policy and has a supervisor, and disclosed in session twenty with a therapist who has never mentioned it, produces two different treatments and two different families.

3.11 Key terms

Confidentiality — the professional duty not to disclose information given in the treatment relationship, subject to specified exceptions.

Privacy — information a person legitimately chooses not to share; compatible with treatment.

Therapeutic secret — information held from one member of a couple or family that materially affects the treatment.

Modified no-secrets policy — a stated policy that individual disclosures will not be kept where they affect the couple or family work, with a process for their disclosure.

Client system — the person or persons agreed to be the client; the holder of the confidentiality.

Third-party claim — an institution's or individual's claim on information, legal or social.

Information map — a drawn record of who holds what information about a family, and who decides.

Deadline — the dated commitment that converts a secret into a task.

Collusion — an arrangement in which the therapist's silence serves the client's avoidance.

Duty to warn / protect — the legal or ethical obligation to act on a serious threat to an identifiable third party; jurisdiction-dependent.

3.12 Self-test

1. Why does individual confidentiality not transfer to couple therapy?
2. State the four parts of the modified no-secrets policy, and the failure each prevents.
3. Give the operational test that distinguishes privacy from secrecy.
4. What is the difference between telling a partner that something has been said and telling them what?
5. In the Adeleke case, why was the health question separated from the confidentiality decision?
6. What would you do if the client refused the deadline?
7. Name five institutions with claims on a family's information, and what each may legitimately be told.
8. Why is "carrying the secret for now" the most damaging option?

Answers. (1) Because the client is the couple jointly, so neither partner can demand confidentiality against the other, and a therapist holding information from one member is deceiving a client on another's behalf. (2) The promise of individual sessions (prevents the request being experienced as a rupture); the limit stated in terms of effect (prevents accidental secrecy); the commitment to tell both that something has been said (prevents the guessing game); the deadline (prevents a stable dishonesty). (3) Does knowing this change what I do in the joint sessions? If yes, it is secrecy. (4) Telling that preserves the discloser's control of the content and the timing, and it converts the therapist's position from deception to pressure; telling what removes the client's only remaining act of repair. (5) Because the exposure of a third party to infection is a health matter with its own urgency, and a therapist absorbed in the ethics of disclosure can lose it. (6) Hold the limit, do not disclose the content, state the date, inform the partner on the date that a disclosure has been made, and keep working with the client so that honesty does not cost her the therapist. (7) The school (attendance and functioning, not therapy content); the court (whatever is compelled); the insurer or employer (usually attendance only); child protection (what the statute requires); the religious body (a summary with written agreement, or nothing); and the extended family, who have no claim and considerable social power. (8) Because there is no "for now": the secret is either moving toward disclosure or it is being carried, and a carried secret changes every question the therapist can ask.

3.13 Reflective task

Think of a time you were told something in confidence that you could not keep from someone who needed to know. Write down what you did, and then answer: which of the six questions did you answer correctly at the time, and which did you answer afterwards? Most people find that the ethical reasoning came after the decision, which is normal — and it is the reason this course insists that the policy is stated before the case arrives.

3.14 Further reading

- Gutheil, T. and Gabbard, G. on secrets in couple and family therapy — the clearest treatment of the triadic problem.
- Margolin, G. on confidentiality in family research and therapy — the empirical discussion of the policy options.
- Doherty, W. and Boss, P. on family ethics — the relational account that underlies this week.
- AAMFT interpretive statements on confidentiality — the shortest authoritative source on the limits.
- Kirmayer, L. on the cultural shaping of disclosure — relevant to the section on family information economies.

Next: Week 4 — Secrets I: the individual disclosure.

Week 4 — Secrets I: the individual disclosure

Learning outcomes. By the end of this week the student can (1) distinguish the ethics of a secret told in individual therapy from the ethics of a secret told in couple therapy; (2) apply a seven-step protocol to an individual disclosure; (3) assess what a secret is doing for the person who keeps it, not only what it is hiding; (4) plan, rehearse and record a disclosure without coercing it; and (5) identify the countertransference that being trusted with a secret reliably produces.

The distinction this week exists to teach

Week 3 dealt with the couple as client. The therapist there had a policy — agreed in advance, applied when the secret arrived — and the policy converted the secret into a task with a date. This week deals with the harder and commoner situation: a person in individual therapy tells the therapist something that concerns someone who is not in the room and is not a client.

The temptation is to import the couple framework wholesale, and it is a mistake. In couple therapy the therapist has a duty to the system, and the system's honesty is a condition of the treatment. In individual therapy the therapist has a duty to one person, and the person's honesty is a condition of the treatment too — but the person's privacy is hers to spend, and the therapist has no standing to demand disclosure to a third party who is not a client at all.

What the therapist does have standing to do is something narrower and more demanding: to insist that the secret be visible inside the therapy. The distinction is the whole of the week.

- The therapist may not tell the husband. He is not a client, has no claim on the file, and telling him would be a plain breach.
- The therapist may not pretend the secret does not exist. A treatment conducted around an unmentionable fact is a treatment of the presenting complaint only, and it will fail in a way that looks like the client's fault.
- The therapist's working question is therefore not should this be disclosed? but what is this secret doing to you, and what would it take for you to be able to put it down?

The classification table. Before responding to any disclosure, classify it. The classification determines what happens in the next five minutes.

Type	Examples	What it obliges
Immaterial	A surprise party; a gift being planned; an ordinary private preference	Nothing. Keep it, and do not make it clinical
Material, no risk	A past termination; a debt; a diagnosis; an affair that has ended; a family history kept quiet	Work with the cost of secrecy; plan disclosure if the client chooses it; never coerce
Third-party risk	An undisclosed infection; a credible threat; a plan to harm	Assess; follow legal and public-health duties; document; disclose only what the duty requires
Child or vulnerable adult	Abuse, neglect, exposure to danger	Report under the statute; inform the client what will be reported
Risk to self	Suicidal intent, self-harm, a plan	Risk assessment and safety planning; break confidentiality only as required

Note what is not on the table: "immoral" is not a category, and "the client's family would want to know" is not a category. The therapist's own moral judgement of the secret is not an input to the classification, and the most common error in this week's work is a classification driven by the therapist's disapproval.

4.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	The individual case: why Week 3's framework does not transfer	Two contrasting vignettes, voted on before analysis
10–25	Five concepts: burden, avoidance, function, countertransference, self-concealment	Lecture with board diagram
25–40	The seven-step protocol; the classification table applied to five disclosures	Live sorting exercise
40–55	The cost ledger and the two-futures exercise	Demonstration with a volunteer, then pairs
55–70	The Bisi case, in full	Case presentation, students hold the decision
70–80	The transcript, annotated	Read aloud in three voices

Minutes	Content	Teaching form
80–90	Six errors; the record; what supervision is for	Short lecture, closing question

4.2 Five concepts that organise the week

Concept 1 — Keeping a secret is work, and the work is paid for in the body. The disclosure literature is consistent on this point: concealment requires continuous vigilance — monitoring what one says, to whom, in front of whom — and that vigilance has measurable costs in physiological arousal, sleep, intrusive thought and sense of authenticity. The clinical implication is not "therefore disclose". It is that the client's reported symptoms may be the cost of the concealment rather than the effects of the concealed event, and that this distinction changes the treatment entirely.

Concept 2 — Secrecy is maintained by avoidance, and avoidance generalises. The client avoids one topic; then avoids the conversations adjacent to it; then avoids the people with whom the topic might arise. In therapy the generalisation is visible as a narrowing of what can be discussed, and it typically looks like a treatment that "has gone flat" rather than like concealment. The therapist's first task is to notice the narrowing, which requires asking about it directly.

Concept 3 — The secret has a function, and it is usually protective. It prevents a divorce, a beating, a loss of standing in a family, a mother's distress. A therapist who treats the secret only as a problem to be solved is asking a client to give up protection before any substitute exists. The function is assessed, named aloud, and taken seriously — and the assessment frequently reveals that the protection is partly illusory.

Concept 4 — Being trusted with a secret produces a reliable countertransference. It is flattering, it creates a special bond, and it invites the therapist to become the client's only ally — which is precisely the position from which the therapist stops challenging her. The signs are familiar: the therapist looks forward to the session, feels protective, has a private understanding with the client, and has begun to dislike the absent husband on the client's evidence alone. Each sign is a prompt to supervision.

Concept 5 — What the client conceals from the therapist matters as much as what she discloses. Self- concealment — the general tendency to hide personal information — predicts poorer outcomes and weaker therapeutic alliances, and it operates on the therapist too. A client who has told you about a past termination may still be concealing the reason she chose you, or the fact that she has already consulted a lawyer. The therapist's own honesty about the limits of what she can work with is the main instrument against this.

4.2a What a secret looks like before it is told

Most secrets are visible for several sessions before they are disclosed, and the therapist's capacity to notice them is a skill that can be trained. The markers below are drawn from the transcript evidence on concealment and from ordinary clinical observation.

Marker 1 — The generic answer. Asked about her marriage, the client who is holding something says "he is a good man" and offers no incident. Detail is where the truth lives; a client who produces adjectives and no events is either not attending to her relationship or not able to report it.

Marker 2 — The topic that never recurs. Every other subject in a client's life will return across six sessions. The one that appears once, in session one, and is never mentioned again, is usually the one that is being managed. The therapist's record should note what is missing as carefully as what is present.

Marker 3 — The time gap. A narrative with a hole in it — school, then marriage, with five years missing — is a narrative with a secret in it. The therapist does not immediately probe the hole. She notes it, and she returns to it when the alliance can bear the question.

Marker 4 — The body's answer. The chest, the throat, the hand that goes to the mouth, the change in breathing when a name is mentioned. These are not proof of anything and they are information, and they belong in the note as observation rather than as inference.

Marker 5 — The abrupt generalisation. The client who is talking about something specific and suddenly becomes abstract ("well, everybody has something they do not talk about") has just come close to it and has stepped away. The move is worth naming gently: "you went somewhere else just then."

Marker 6 — The pre-emptive question. "You are not going to tell anyone what I say, are you?" — asked before anything has been said, and usually asked more than once. The client is not testing the policy; she is preparing to use it.

How the therapist uses the markers. She does not accuse. She makes the space available: "is there anything you have been carrying that we have not got to?" — asked once, plainly, and then left. The question is an invitation and not a demand, and the client who declines it has learned that the door is open, which is what makes the later disclosure possible.

4.3a When the client is a child or an adolescent

The protocol changes, because the client's capacity to choose is limited and the parents' claim is real.

The default. A minor's confidentiality is real and is not absolute. The therapist tells the child, in the first session and in language appropriate to the age, what will be shared with parents and what will not. The standard formulation: "what you tell me stays with me, unless I think you or someone else is in danger, or unless I think you need help that only your parents can give — and if I ever have to tell them something, I will tell you first what I am going to say."

The common error. Promising a child total secrecy, which the therapist cannot keep and which, when broken, teaches the child the exact lesson the therapy exists to prevent.

The adolescent's material. An adolescent disclosing sexual activity, a relationship, or a plan to leave home is disclosing material that parents will want and frequently have a legitimate interest in. The therapist's position: the content stays confidential where there

is no risk; the fact that the young person is dealing with something significant may be shared with the parents in general terms, with the young person's knowledge. Week 2's four-part script for consent with a young person is the template.

Where the child holds a family secret. A nine-year-old who says "do not tell my mum that daddy shouts" is disclosing something about the household. The therapist assesses risk first, then asks the child what she is most afraid would happen if it were known, then works with the parents on the pattern without attributing it to the child. The child must not be made the source, because a child who is the source of a parental crisis has been given a burden that will outlast the therapy.

4.3 The seven-step protocol for an individual disclosure

Step 1 — Receive it without reacting. The most consequential fifteen seconds in the session. The client has taken the largest risk she will take in months, and she is reading the therapist's face for the verdict. Any visible judgement — surprise, disapproval, concern so sharp it reads as alarm — teaches her that the disclosure was a mistake. The therapist's response is small and warm: "thank you for telling me", a pause, and then a question that keeps her talking.

Step 2 — Classify. Run the table. Is anyone at risk? Is a child involved? Is a duty engaged? This takes seconds and can be done while the client is still speaking. If any risk category applies, Week 8 or 9 governs and the session changes shape immediately.

Step 3 — Establish the history of the secret. How long has she held it, who else knows, has she ever tried to tell, and what happened. This is not small talk. The answers tell the therapist whether she is dealing with a long-maintained concealment (which will have shaped her relationships) or a recent event (which will not have had time to).

Step 4 — Assess the function. The question is: "what does keeping this do for you?" The answers are specific and are usually accurate — it protects her marriage, her mother, her standing, her job. The follow-up is the one that does the work: "and what does it cost you to keep it?" The client has almost always done this accounting privately and has never been asked to say it aloud.

Step 5 — Name the cost without demanding the remedy. The therapist reflects what she has heard, makes the link between the concealment and the presenting symptoms, and then does not prescribe. "So the sleep, and the knot in your chest — I wonder whether they are the price of carrying this alone." The client is now holding a formulation of her own problem rather than a secret.

Step 6 — Offer options, and let her choose. Five, in ascending order of exposure: keep it and work on managing it; tell one other person outside the marriage; tell the husband with the therapist's help in planning; tell the husband in a session with the therapist present; or defer with a review date. The deferral option is essential: without it, the client who is not ready has only the choice between exposure and flight.

Step 7 — Record and review. The note records the classification, the reasoning, the client's decision, and the review date. The review is a real appointment, and the therapist raises it.

What is absent from the protocol. Any step at which the therapist decides for the client. This is the difference from Week 3, and it is not a difference of squeamishness: the therapist has no authority over the client's relationship with her husband, and the attempt to exercise it converts treatment into moral direction.

4.4 Four techniques

Technique 1 — The cost ledger. A two-column sheet. Left: what keeping the secret protects. Right: what it costs — sleep, intimacy, the topics that cannot be raised, the moments of panic at a family gathering, the doctor's question that cannot be answered honestly. The client fills it at home between sessions. The sheet does the work that a therapist's argument cannot do, because the client produces both columns herself. In practice the left column is short and stable and the right column is long and growing, which is the finding that shifts the decision.

Technique 2 — The two futures. A structured imagination exercise, ten minutes, eyes open. The client describes in detail the morning three months after she has told him: where she is, what the room looks like, what he said, what happened to her body, and what she does next. Then the same morning in three years if she has not told him. The exercise is not a prediction and is not offered as one. Its function is to convert a vague dread into a set of specific fears, each of which can then be examined — and frequently to reveal that the feared outcome that has kept her silent for eleven years has never once been described in detail.

Technique 3 — The smallest true sentence. Many clients cannot contemplate the disclosure because they are imagining a full confession. The technique constructs the smallest sentence that is true and complete: "there is something about my past I have never told you and I want to tell you." Eleven words. The rehearsal is of these eleven words and the silence after them, and nothing else — because the client's real fear is not the content but the moment before it, and the moment can be practised.

Technique 4 — The audience question. "Who, other than your husband, would this change things with?" In a family where a mother-in-law, a pastor, or a brother has a stake, the client is often protecting the secret from four people while nominating one. Naming the full audience reduces the fantasy to a specific list and sometimes shows that the disclosure is smaller than she thinks — or larger.

A note on rehearsal. Where a client decides to disclose, the therapist rehearses with her: the opening sentence, the two most likely responses, and the sentence she will use if the conversation goes badly. The rehearsal is not preparation for a performance; it is the demonstration that she has options inside the conversation, which is what fear removes.

4.4a The case: Bisi Oyelaran, and what the therapist did not do

Referral. Bisi Oyelaran, 29, a primary school teacher, self-referred for "sleep and worry". Two children, 3 and 1. Husband Dayo, 33, a bank officer. Married four years. No prior therapy.

Sessions 1–4. The presenting problem is insomnia, a sensation of pressure in the chest, and irritability with the children. There is no identifiable stressor of the usual kind: work

is stable, the marriage described as "fine", the children healthy. Bisi's mother died two years earlier and the therapist pursues bereavement, which produces some material and no improvement. In session 4 the therapist notices the pattern the reader should have noticed in session 2 — every account of the marriage is generic. "He is a good man" recurs five times. There is no anecdote, no argument, no detail.

The moment, session 5, minute twenty-two. "I have to tell you something and then I will probably not come back." The therapist says nothing except "I am listening." Bisi then discloses: at 21, in her final year at college, she was pregnant and had a termination. Her mother knew and arranged it. No one else has ever known. Dayo's family are members of a church where this is "the worst thing a woman can do", and his mother has begun asking when the third child will come, and has suggested a fertility test.

The classification, made in the moment. No risk to anyone. No child protection question. A material secret with no third-party risk. The protocol applies.

Step 1 in practice. The therapist said: "thank you for telling me, and I want you to know that I am not going anywhere." Bisi cried for four minutes. Nothing was said. This is not a failure of technique; it is the technique.

Step 3, the history. Eleven years. Two people knew — Bisi and her mother, and her mother is dead. Since the death, no one knows. The therapist names this: "so for two years you have been the only person in the world who knows this." Bisi said, "yes", and then, "I had not thought of it like that."

Step 4, the function and the cost. The function: it protects the marriage from Dayo's family, and it protects her from Dayo's judgement, which she describes as "he would not shout, he would go quiet, and then he would tell his mother." The cost, elicited over two sessions: she cannot answer questions about her twenties; she avoids her college friends; she has refused two invitations to speak at church; she has begun to dread intercourse because "it is the one place I think about it"; and the chest pressure began the week her mother-in-law first mentioned the fertility test.

Step 5, the naming. "You have been carrying this alone for two years, and the fear of being found out has started to organise your life — who you see, what you say, even when you are close to your husband. I do not think the sleep is the problem. I think the sleep is the bill."

Step 6, the options, offered without pressure. Bisi rejected disclosure to Dayo outright, and said so with contempt for the suggestion. The therapist did not argue. Bisi chose a version of the first option — keep it, work on managing it — with a review date in six weeks, and one addition of her own: she wanted to tell one person, a cousin in Ibadan, "so that somebody alive knows".

The decision the therapist did not make. The therapist did not tell Bisi that she must tell her husband, did not imply that a good marriage requires it, and did not treat the delay as a failure of courage. She said what she thought: that the secret was costing her more each year, that the cost would rise when the fertility question became a medical

process, and that if Bisi ever wanted to tell him she would help her plan it and would be in the room if wanted.

Sessions 6–12. The work changed. The presenting symptoms were reframed as the cost of concealment, and the treatment became about how Bisi managed the secret rather than about the secret. She wrote the cousin a letter she did not send, then sent it. The chest pressure reduced over five weeks. In session 10 she said, of her own accord, "I think I will tell him before the test", and the therapist said "tell me what you would want to say", and the rehearsal began. She told him in session 14, at home, and came to the next session with the two of them. The therapist then saw them twice as a couple — with a new consent conversation, because the client system had changed.

What the case teaches.

1. **The therapist's own urgency is the main threat to the work.** Had she pressed in session 5, Bisi would have left, and the secret would have gone back underground with a new piece of evidence: that telling costs you the person you told.
2. **The symptom was the bill.** Reframing the presenting complaint as the cost of concealment is the single most useful intervention in this week's material, and it is available without any disclosure at all.
3. **The disclosure was eventually made, and not because it was required.** It was made when the cost of the secret exceeded the cost of the telling, and the therapist's job was to make that comparison visible and to be present when the balance tipped.
4. **The client system changed, and the paperwork had to change with it.** When Dayo joined, the consent conversation was redone from the beginning, and the record was updated. A therapist who simply continues on the old consent has made a documentary error that no amount of good clinical work repairs.

4.5 Transcript: receiving the disclosure

Session 5, minutes twenty-two to forty. Bisi has just said "I have to tell you something and then I will probably not come back." Annotation after each turn.

> **T1 Therapist:** (pause) I am listening. — Four words. No question, no reassurance, no "of course you will come back". The client has announced an exit; the therapist does not argue with it, which is what keeps her in the room. **T2 Bisi:** (long pause, then quickly) When I was twenty-one, in my final year, I was pregnant. My mother took me. I have never told anybody. Dayo does not know. His family — you do not know his family. If they knew they would — (stops) **T3 Therapist:** Thank you for telling me. (pause) And I want you to know that I am not going anywhere. — Step 1. The two sentences are doing different jobs: the first acknowledges the risk she has taken; the second answers the fear she actually voiced, which was about being abandoned by the therapist, not about the termination. **T4 Bisi:** (crying) You are not going to say anything? **T5 Therapist:** Not yet. (pause) I would rather hear the whole of it than start talking. — The refusal to reassure is the hardest move in the transcript and the most important. Reassurance at this point ("that was not your fault", "God forgives") replaces the client's account with the therapist's, and the client learns that the subject must be managed. **T6 Bisi:** There is nothing else. It was eleven years ago. It is finished. **T7 Therapist:** Eleven years. (pause) Who knows? **T8 Bisi:** My mother. Only my mother. **T9 Therapist:** And your mother died two years ago. — Step 3. The therapist has done the arithmetic and says it plainly. The client's face changes; this is the first moment in which the isolation is visible to her. **T10 Bisi:** (pause) Yes. So now it is only me. **T11 Therapist:** (pause) So for two years you have been the only person alive who knows this. — The reflection that does more than any interpretation. It is accurate, it is short, and it is not a judgement. **T12 Bisi:** (long pause) I did not think of it like that. I thought — I thought I was managing it. **T13 Therapist:** You have been managing it. (pause) Tell me what managing it looks like. — The question that opens the cost ledger without the therapist having to propose it. **T14 Bisi:** I do not talk about university. If somebody asks, I say it was fine. I do not go to the reunions. I stopped singing in the choir because — (stops) — because I thought if I stood up there, everybody would know. **T15 Therapist:** Everybody would know. — Reflecting the exact phrase, which is the technique that gets the client to hear her own logic. The therapist does not correct it. The correction is available later and is worth nothing now. **T16 Bisi:** I know it is not reasonable. **T17 Therapist:** I did not say it was not reasonable. I said I heard it. — A small correction, and a necessary one: the client has begun to argue with herself, and self-argument is the mechanism by which the secret has been kept. **T18 Bisi:** (pause) His mother has started talking about a test. She wants to know why we are not expecting. And I sit there and I — (stops) **T19 Therapist:** And you sit there holding all of it. — The link between the secret and the presenting symptom, made in the client's own language, four minutes after the disclosure and without a formulation. **T20 Bisi:** (quietly) The chest started then.

Minute forty, the close. The therapist did not offer a plan in this session. She said: "This is a great deal to have said aloud, and I think we should stop here rather than start solving it. I want you to come next week. We will look at what you want to do about this — and there are several things you can do, not one." Bisi attended the following week.

What the transcript shows.

- **Restraint is the intervention.** Of the twenty turns, the therapist asks four questions and makes three reflections. She interprets nothing, reassures nothing, and prescribes nothing.

- **The exact phrase.** Reflecting the client's own words (turns 15 and 19) produces more than any formulation the therapist could have offered, because the client cannot argue with her own sentence.
- **The ending is deliberate.** Closing before problem-solving prevents the session from becoming a negotiation about disclosure in the first hour of the client's exposure, and it makes the next appointment the container for the decision.

4.6a The record of an individual disclosure, written out

Two notes, from the Bisi case, at the two moments that matter. The first records the disclosure; the second records the decision at review.

> **SESSION 5 (50 MIN).**
> Client disclosed a termination at age 21, during her final year at university. Her mother arranged and knew; mother died two years ago; no other person has known. Client states her husband and his family do not know and that she fears disclosure would end the marriage and expose her to her mother-in-law and her church community. **Classification.** Material, no current risk to client, child, or third party. No statutory duty engaged. No risk indicators on direct enquiry. **Response.** No advice or plan offered in this session; disclosure received, history and audience explored, link noted between the client's reported chest symptoms and the recent fertility questions from her mother-in-law. Session closed early by design; next appointment confirmed and attended. **Plan.** Options to be discussed at session 6 (keep and manage; tell one other person; tell husband with planning; tell husband in session; defer with review). Client informed there are several options, not one. **Supervision.** Case to be presented [date] — countertransference noted: therapist aware of a wish to protect client from her husband's family.

> **SESSION 11 (50 MIN).**
> Review held as scheduled. Client reports: has told one person (cousin, Ibadan); chest symptoms reduced; sleep improved from approximately four to six hours. Client states she now intends to tell her husband before any fertility investigation begins, and asks for help planning. Rehearsal begun with the opening sentence; two likely responses identified; client to bring a written note to session 12 if she wishes. Client informed that if she wishes the therapist to be present, the therapist will attend; client has not yet decided. **Client system unchanged at this date.** Review at session 13.

What the two notes have in common. Both record the classification, the reasoning, the client's decision, the next step and the supervision. Neither contains a judgement of the client's choice. Neither records an instruction.

Why the second note matters as much as the first. It is the evidence that the deferral was a plan and not an abandonment. A file containing the first note alone tells a reader that the therapist knew about a secret and did nothing for six sessions; the two notes together tell the truth, which is that the client was given time, the work continued, and the decision was hers.

The one sentence that must never appear. "Client declined to disclose; discussed." It is accurate, it is useless, and it will be read by a regulator as an absence of clinical thinking.

4.6 Laboratory: the disclosure rehearsal

Aim. To practise receiving a disclosure without reacting, classify it correctly, and offer options without coercion — assessed by an observer against six observable behaviours.

Set-up. Triads, three rounds of twelve minutes each, so everyone plays all three roles. Each round the "client" draws a card. The cards, which should be prepared in advance:

Card	Disclosure	Classification
A	"I have been paying a loan for two years and my husband does not know."	Material, no risk
B	"I have not told my wife that I lost my job six weeks ago."	Material, no risk; possible financial risk
C	"My brother has been staying with us and he has been in prison."	Material; consider third parties
D	"I have been having thoughts about not being here."	Risk to self — stop the exercise and follow Week 8
E	"My husband's cousin touched me when I was sixteen and I have never told anyone in the family."	Material; consider historical abuse reporting rules
F	"I know my daughter's husband hits her and she has asked me not to tell anyone."	Risk to a third party — Week 9

Cards D and F are the ones that matter pedagogically: the student who continues the counselling protocol after drawing them has failed the round outright, however warm the response.

Observable behaviours (the observer marks each as present, partial or absent).

1. Did the therapist allow a silence after the disclosure, before speaking?
2. Did the therapist thank the client, and say something that addresses abandonment?
3. Did the therapist ask about history and audience before proposing anything?
4. Did the therapist ask about function and cost?
5. Did the therapist offer at least three options, including a deferral with a review date?
6. Did the therapist avoid reassurance, advice, and any statement of what the client "should" do?

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Reception	Reacted, advised or reassured	Paused, but then filled the silence	Received without reaction; acknowledged risk; addressed abandonment
Classification	Did not classify	Classified, but the wrong category	Correct category, and changed protocol where the card required it

Criterion	0–1	2–3	4–5
Function and cost	Neither explored	One explored	Both explored in the client's language, with the link to the presenting complaint
Options	Gave one option or a demand	Several, all leading to disclosure	Several, including deferral and review, with the choice left where it belongs
Record	Nothing	Narrative only	Classification, reasoning, client's decision and review date

Debrief question for every triad. "What did you want the client to do, and where did that wish show in what you said?" Almost every student discovers the wish in the pause before their first question.

4.7 Six errors

Error 1 — Reassuring in the first minute. "That must have been terrible", "you were very young", "God is merciful" — each is meant kindly and each takes the story away from the client. The client's task at that moment is to finish her account; the therapist's task is to make room for it.

Error 2 — Classifying by the therapist's moral reaction. The therapist who finds an affair or a termination disturbing will classify it as high-priority and press for disclosure. The classification table has no moral criterion, and its absence is deliberate.

Error 3 — Turning the first session of disclosure into a decision. The client has spent years holding something and has now said it aloud, which is exhausting. A therapist who wants a plan within twenty minutes is working at her own pace, not the client's.

Error 4 — Confusing "I cannot work around this" with "you must tell". The therapist's limit is about her own capacity to conduct an honest treatment, and it is legitimate. It is not a demand that the client disclose to a third party who is not a client, and the two are frequently conflated.

Error 5 — Becoming the keeper of the secret. The therapist who agrees to hold it, protects the client from the husband, and begins to manage the household's information has been recruited. The sign is that the therapist is now thinking about the husband as an obstacle.

Error 6 — Forgetting to review. The deferral option is safe only if the review is real. A deferral without a date becomes a permanent arrangement, and the secret has quietly been installed in the treatment.

4.4b Four sentences that help a client decide

The therapist's most useful contribution to a disclosure decision is not advice; it is a small number of sentences that make the decision thinkable. These four, used in this order across two sessions, do most of the work.

First — "you do not have to decide today." The client has usually arrived at the session believing that telling the therapist means being pushed. Removing the pressure lowers the arousal enough for thinking to become possible, and it is also true.

Second — "there is more than one way to do this." The client's imagination contains a single scenario: sitting opposite her husband, saying everything, and watching the marriage end. The therapist's job is to populate the field — a letter, a session, a trusted third person present, a partial disclosure of the fact rather than the detail, a staged disclosure over weeks. Clients almost never generate these options alone, because fear narrows the field to one.

Third — "what is the worst thing you are imagining, in detail?" Not "how do you feel?" — which produces adjectives — but a request for the specific scene. The scene is then examined: what he says, what she says next, who else is told, what happens in the following week. The examination does two things. It usually reveals that the imagined catastrophe has no script beyond the first minute. And where the catastrophe is real — where the husband has been violent, or where the family would remove the children — the examination reveals that too, and the plan changes accordingly.

Fourth — "whatever you decide, I will still be working with you." The sentence that makes honesty survivable and the one most often left unsaid. It is also the sentence the therapist must be able to keep, which is why the limit in Week 3's couple policy is different from the position here.

4.7a The audit question, and the end of the week

At the end of every week in which a disclosure has been received, the therapist answers four questions in writing, in the file, in under a hundred words.

1. **What do I now know that the people around this client do not know?** The inventory. It is usually longer than the therapist expects.
2. **Is anything moving, or is this now installed?** A plan with a date is moving. A deferral with a review date is moving slowly. A deferral without a date is installed.
3. **Who is paying for the concealment, and how much?** If the answer is only the client, the situation is stable. If the answer includes a child, a partner's health, or a third party's exposure, the situation is not stable and the deferral must be renegotiated.
4. **Have I taken this to supervision?** If not, why not — and the honest answer is frequently that the therapist does not want to be told to press, which is itself the material for supervision.

The closing point of the week. The student who finishes this week believing that the right answer is "the client should disclose" has not learned the week's content. The right answer is that the therapist maintains a relationship in which the client can tell the truth about what she is carrying, keeps her own conduct honest, never becomes a party to the concealment, and holds the client's choice even when the choice is to wait. That is a harder position to occupy than either directive — and it is the position the codes describe when they say that the therapist's duty is to the client's welfare and self-determination, and not to the therapist's view of the client's marriage.

4.8 Culture, faith and the secret

Shame is social before it is private. In many of the families this book serves, a secret is not held to protect the individual's self-image but to protect a household's standing. Bisi's fear was not that Dayo would think less of her; it was that Dayo would go quiet and then tell his mother, and that the information would then travel through a church and a compound. The therapist who assesses the risk as "her husband's reaction" has mis-specified it, and the mis-specification makes the client's fear look irrational when it is not.

Confession has an address. Where a client's framework includes confession — to a priest, a pastor, an imam, an elder — the therapist should ask, early, whether the client has confessed, and to whom, and what happened. Three outcomes are common and each is clinically useful. The client has confessed and found absolution, and the secret's burden is much lower than the therapist assumed. The client has confessed and been given a penance that made the secrecy worse, such as a direction never to speak of it again. Or the client has not confessed because the confession would require a public act she cannot perform, and the therapy is now carrying a religious obligation as well as a personal one.

The church as an information network. In congregations where testimony is public and pastoral care is layered through small groups, a disclosure has an audience that the client knows better than the therapist. The practical consequence: ask who would come to know, and do not assume the answer is one person. In Bisi's case the audience was a mother-in-law, a women's fellowship, and a choir.

The in-law's claim. In many households the mother-in-law holds a genuine and socially recognised claim on information about fertility. A therapist who treats this as intrusion will alienate the client, who may agree with the claim even while resenting it. The workable formulation is to name the claim as real and to separate it from the decision: "she has a claim to be told something; she does not have a claim to be told this, and the choice of what she is told is yours."

Reproductive history and the honour frame. Where a premarital pregnancy or a termination is framed as an offence against family honour rather than as a personal decision, the disclosure's danger is not disproportionate — it is accurately perceived. The therapist's obligation is not to talk the client out of the danger but to help her assess it with precision and to plan for the realistic worst case, including where she would go and who would take her in.

What not to do. Do not import the language of "authenticity" and "living your truth" into a household where the cost of disclosure may be housing, custody, or violence. Do not assume that the Western therapeutic consensus — disclosure is good for you — describes the client's situation. And do not assume the opposite either: many clients in these households disclose, survive it, and are relieved.

4.8a The therapist's own secrets

A neglected part of the topic, and a frequent source of impairment.

The therapist is not a sealed vessel. Therapists hold their own material — a divorce, a miscarriage, a parent's illness, a history of abuse, a child in difficulty — and the case that resembles it will activate it. The professional obligation is not to be unaffected. It is to know what is activated, to keep it out of the client's material, and to take it to supervision.

The three failure modes. Over-identification: the therapist's own secret shapes what she hears, so that the client's situation becomes the therapist's situation and the client's options narrow to the ones the therapist chose. Compensatory rescue: the therapist protects the client from the outcome she herself suffered. Displacement: the therapist's unprocessed material appears in the case as unusual intensity — the session that runs over, the client she thinks about at night, the case she discusses with her own partner.

Self-disclosure as a clinical decision. When the therapist discloses something of her own, the criteria are narrow: it must serve the client's work, it must be brief, it must be about the therapist's experience rather than the client's, and it must not require the client to take care of the therapist. "I have sat with this kind of decision myself, and I know it is not simple" meets the criteria. A twenty-minute account of the therapist's own divorce does not, and it makes the client a witness rather than a patient.

The question for the therapist. Before each session with the client who activates you: whose material is this? If the answer is unclear, the case goes to supervision and, where the activation is severe, the therapist considers whether the case should be transferred — which is a professional act and not a confession of inadequacy.

4.9 Evidence note

What the research supports. The concealment literature is substantial. Experimental work finds that holding a secret produces intrusive thinking about it and consumes cognitive capacity; that keeping secrets is experienced as effortful and fatiguing; that self-concealment is associated with greater distress and poorer help-seeking; and that disclosure interventions produce modest but reliable improvements in health and wellbeing. In psychotherapy research, clients' concealment predicts weaker alliances and worse outcomes, and client-rated honesty about what they have not said is a better predictor of progress than the amount they have said.

What is weaker. The evidence is largely about the cost of concealment in general, not about the right moment to disclose to a spouse. There are no trials of disclosure timing in couple or family treatment, and the prescriptive literature — tell within X sessions — is clinical convention rather than evidence.

What is not supported. That disclosure is always beneficial: the literature is clear that disclosure to a hostile or unsupportive recipient can worsen outcomes, which is precisely the situation many clients in this week's material accurately fear. That a therapist can predict the outcome of a disclosure. And that a secret unspoken to the therapist cannot be worked with — the treatment can address the management of a secret without its content, and sometimes must.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Bisi at 21, in a family where her

mother could act, and Bisi at 29, with two children, a mother-in-law and a church, is the same event in two different systems, and the disclosure carries two different prices.

4.10 Key terms

Individual disclosure — information given in individual therapy concerning a person who is not a client.

Concealment cost — the practical and physiological price of maintaining a secret.

Function — what the secret protects; assessed before any plan is proposed.

Self-concealment — a general disposition to hide personal information; predicts poorer therapy outcomes.

Deferral with review — an agreed postponement with a dated review; the alternative to disclosure and flight.

Smallest true sentence — the shortest complete disclosure, used in rehearsal.

Cost ledger — a two-column client exercise weighing protection against price.

Countertransference to the secret — the therapist's protective, flattered or allied response to being trusted.

Client system change — the addition of a new client (a partner, a child) requiring a new consent conversation.

4.11 Self-test

1. Why does the modified no-secrets policy of Week 3 not transfer to individual therapy?
2. State the five categories of the classification table and give one example of each.
3. What are the three signs that a therapist has been recruited as keeper of the secret?
4. Describe the cost ledger and explain why the client fills it rather than the therapist.
5. What is the smallest true sentence, and what is it for?
6. In the Bisi case, what did the therapist not do, and why does each omission matter?
7. What must be recorded after an individual disclosure, in five parts?
8. What changes when a new client joins an existing treatment?

Answers. (1) Because the client is the individual, so the therapist owes confidentiality to one person and has no standing to require disclosure to a non-client; the therapist's legitimate limit is on her own capacity to conduct the treatment, not on the client's conduct. (2) Immaterial (a surprise party); material with no risk (a past termination); third-party risk (an undisclosed infection); child or vulnerable adult (abuse); risk to self (suicidal intent). (3) The therapist looks forward to the session unduly; she feels protective of the client against the absent partner; and she is managing the household's information. (4) A two-column sheet of what the secret protects and what it costs, completed at home, because the client's own production of the right-hand column is more persuasive than any argument the therapist could make. (5) "There is something about my past I have never told you and I want to tell you" — eleven words used in rehearsal, because what the client

fears is the moment before the disclosure rather than its content. (6) She did not reassure in the first minute (which would have replaced the client's account with her own); did not propose a plan in the first session (working at the therapist's pace); did not tell Bisi she must disclose (which would have ended the therapy); and did not treat the delay as failure. (7) Classification; reasoning; the client's decision; the review date; and the supervision plan. (8) A new consent conversation, an amended confidentiality agreement, a new record of who is the client, and a re-negotiation of what will be shared between the individual and joint work.

4.12 Reflective task

Write two hundred words on the last secret you were told. Do not describe the content. Answer three questions: what did you feel towards the person who told you; what did you want them to do next; and did that wish appear anywhere in how you responded? Keep the page. You will want it when the first case in your practice reproduces the feeling.

4.13 Further reading

- Pennebaker, J. on disclosure and health — the empirical basis for the cost-of-concealment formulation.
- Slepian, M. on the psychology of secrecy — the experimental work on mind-wandering and concealment burden.
- Larson, D. and Chastain, R. on self-concealment — the construct that predicts therapy process.
- Doherty, W. on moral conflicts in therapy — relevant to the discipline of not moralising.
- Any current text on professional ethics in counselling for the treatment of client concealment from the therapist.

Next: Week 5 — Secrets II: infidelity, paternity and HIV status.

Week 5 — Secrets II: infidelity, paternity and HIV status

Learning outcomes. By the end of this week the student can (1) conduct the first four sessions after the discovery of an affair without either minimising the betrayal or treating the marriage as finished; (2) sequence the clinical and legal tasks when paternity is in question; (3) identify the jurisdiction's rule on disclosure of a communicable condition and follow the correct pathway where a partner is at risk; (4) decide, with reasons, whether the affair partner's spouse should be told; and (5) document all three in a form that would survive a regulator's reading.

Three secrets, three legal overlays

Week 4 dealt with the individual secret in general. This week deals with three specific secrets that arrive with predictable regularity and that each carry a legal overlay the therapist must recognise — not to practise law, but to know when to stop and refer.

Secret	Clinical task	Legal overlay	Who else is exposed
Infidelity	Contain the disclosure; treat the betrayal; rebuild or end	Not an offence in most jurisdictions; relevant to divorce, custody and maintenance	The affair partner, and the affair partner's spouse
Paternity	Support the family through a factual discovery with enormous relational meaning	Presumption of paternity, maintenance, custody, registration of birth, the child's rights	The child, the presumed father, the biological father, both extended families
HIV and com municable disease	Support disclosure; treat the fear, the shame and the couple's sexual negotiation	Public-health law; in some jurisdictions a criminal offence to transmit or to expose knowingly	The partner, and any previous or subsequent partner

The three are grouped because they share a structure: an **established fact**, a **person who does not know it**, and a **third party who may be harmed by the continuing silence**. Week 3's policy governs how the therapist holds the secret; this week governs what the therapist does with it.

The first thing to establish, always, is jurisdiction. The single most common ethical error in this week's material is a therapist applying a rule learned in one country to a family in another. On paternity, the presumption of legitimacy and the rules on registration differ substantially. On HIV, some jurisdictions impose a duty to disclose to an identifiable partner, some impose a duty to warn a public health authority, some criminalise non-disclosure, and some do none of these. On infidelity, adultery remains a criminal offence in a number of jurisdictions and is a ground for divorce in most. The therapist's obligation is to know the local rule for the three cases, to have checked it within the past year, and to have it in writing in the practice manual — not to reconstruct it from memory in a session.

The second thing is to know what the therapist does not do. She does not advise on divorce, on custody, on maintenance, on the registration of a birth, or on whether a client should be tested. She does not tell a client what the law requires of the client — that is legal advice, and where the stakes are this high it must come from a lawyer. She may say what her own duties are, because those are her duties and the client is entitled to know them. The distinction is clean and it is frequently blurred by therapists who have read one article.

5.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The three secrets and the common structure	Board diagram: fact, unknowing person, exposed third party
8–20	Five concepts: betrayal trauma, the disclosure scene, the legal overlay, exposure, the third party's claim	Lecture
20–38	Infidelity: the four-session protocol	Demonstration, then pairs on the opening sentence

Minutes	Content	Teaching form
38–52	Paternity: the sequence, and where the therapist stops	Case vignette, students write the referral question
52–66	HIV: jurisdiction, pathway, and the safe disclosure plan	Local rule quiz; group mapping of the disclosure pathway
66–76	The Adegoke case and the disclosure session	Case presentation with students as the couple
76–86	Six errors; the documentation	Short lecture
86–90	Closing question: whose secret is it?	Written answer, collected

5.2 Five concepts that organise the week

Concept 1 — Betrayal trauma is a trauma, and it is not a metaphor. The discovery of an affair produces a response with the signature of traumatic stress: intrusive images, hypervigilance to cues, avoidance, sleep disruption, and a compulsive need for detail. This matters clinically because the therapist who treats the discovery as an understandable upset will under-treat it. The partner who has just found out is not merely angry; she is in an acute stress response, and the first task is stabilisation rather than communication training.

Concept 2 — The disclosure scene is the treatment's foundation, and it happens once. The first conversation in which the affair is discussed will be remembered verbatim by both partners for years. What is said, whether the details are given, whether the affair partner is named, and whether the therapist is present or absent determines the shape of everything that follows. The therapist therefore prepares this conversation as carefully as any intervention in the book.

Concept 3 — The legal overlay changes what can be promised. A therapist who promises a client that nothing said in the room will reach a court has made a promise she may not be able to keep, because records can be subpoenaed and because in some jurisdictions the therapist's notes are discoverable in custody proceedings. The correct formulation, given at the start: "what we say here is confidential, and there are limits — one of them is that a court can compel records. If that ever happens, I will tell you and I will fight for as much confidentiality as the law allows." (Week 11 develops this.)

Concept 4 — Exposure is the therapist's signature question. Not "what should the client do?" but "who is exposed by the continuation of this silence, and how serious is the exposure?" An undisclosed affair exposes the affair partner's spouse to the same betrayal and, if there is any sexual contact outside the primary relationship, to infection. An undisclosed paternity exposes a child to a factual discovery later, at an age when it is more damaging. An undisclosed HIV status exposes a partner to infection. The seriousness of the exposure determines how much urgency the therapist brings.

Concept 5 — The third party has no claim on the therapy and a real claim on the truth. The affair partner's spouse is not a client, has no right to the file, and cannot be contacted by the therapist without the therapist's own breach of confidence. She also has a genuine moral claim to information that affects her marriage and her health. The two

facts are both true, and the therapist must hold them without resolving the tension by pretending one is absent.

5.2a Infidelity: the four-session protocol

Session one — stabilise, do not adjudicate.

- Both partners are seen, usually separately first, for twenty minutes each, then together briefly.
- The injured partner is assessed for acute stress: sleep, intrusiveness, functioning, and suicidal ideation, which is not rare after discovery.
- The involved partner is assessed for the character of the affair: how long, how often, whether it is ongoing, whether there is a pregnancy, whether money has moved, and whether the affair partner is threatening disclosure.
- The therapist does not ask "why did you do this?" in session one. The question invites a defence that then has to be dismantled, and it is the wrong question at the wrong moment.
- The therapist states the frame: no decisions about the marriage for six weeks; individual sessions permitted, with Week 3's policy; and the disclosure conversation will be structured, not improvised.

Session two — the disclosure scene.

Structure it as five questions, answered by the involved partner, in this order, with the therapist controlling pace:

1. What happened? The facts, in outline — when it began, when it ended, how often. Not sexual detail.
2. Who knows? Including the affair partner's spouse, and any friend who has been told.
3. What did you tell me about it? The lies told to the injured partner during the affair, named specifically. This is the part that is hardest for the involved partner and the most necessary for the injured one, because the injured partner's injury is not only the sex; it is the deception she participated in unknowingly.
4. What are you going to do now? Continuation, ending, disclosure of the affair to the affair partner's spouse, and the practical steps — the message sent, the block, the change of arrangements.
5. What do you need from me? Addressed to the injured partner; the answer is usually "I need it to stop", and frequently "I need to know you chose me".

The detail question. The injured partner will ask for details, and the answer is not "tell everything" or "tell nothing", it is the distinction between facts necessary for safety and choice and images that will not leave. Necessary: how long, whether protection was used, whether there is an ongoing contact, whether the affair partner has been told about the marriage. Not necessary, and harmful: positions, comparisons, what the other person looks like. The therapist says this to both of them, before the questions begin, and both partners usually accept it — because the injured partner's real question is not anatomical.

Session three — the meaning.

Only now, after the facts are held and the exposure has been managed, does the question "what was this about?" become answerable. The therapist's stance must be exact: the meaning is not a cause, and the cause is not an excuse. "I want to understand what was happening in your life when this started, and I am not asking so that we can blame your wife for it. I am asking because whatever made the affair possible will still be there when it is over." The distinction is the one thing that makes the third session survivable, and it must be said aloud.

Session four — the choice.

The marriage is named as a decision rather than a default. Both partners are asked, separately and then together, whether they intend to attempt repair. Where one partner will not, the therapist does not argue; she helps them separate without the affair becoming the whole of the story. Where both will, the treatment moves to the repair tasks: full transparency for a defined period, the naming of what was lost, the rebuilding of trust as a set of testable commitments rather than a feeling, and the explicit rejection of "we are past it now" as a route to a silent recurrence.

The temporal frame that matters. The literature on affair recovery converges on a rough shape: an acute phase of three to six weeks in which nothing is decided; a middle phase of several months in which the meaning is worked and trust is rebuilt through consistency; and a long tail in which anniversaries and discoveries of new detail re-open the injury. A therapist who promises the couple that "in twelve sessions you will be through this" has promised something false and has given them a schedule to fail against.

5.2b Paternity: the sequence, and where the therapist stops

The situation. A father learns, or suspects, that a child he is raising is not biologically his. Or a mother learns that the man she named as the father is not. Or an adult child learns, from a commercial DNA test, that her father is not her father. Each of these arrives in a therapist's office and each is treated by the therapist as a family disclosure with a legal dimension.

Step 1 — Establish what is known, and how. Suspicion, an admission, a test, or a document. A suspicion the client has carried for nine years is not the same fact as a test result, and the clinical work differs enormously. The therapist's first question, "what do you actually know, and how do you know it?", is answered incompletely about half the time.

Step 2 — Establish who knows. The mother, the man, the child, the extended family, a lawyer. In the commonest configuration, two adults know and a child does not, and the therapist's client is one of the two.

Step 3 — Establish the child's position. Age, developmental stage, relationship with the presumed father, relationship with the biological father if any, and what the child has been told. The child's welfare is the organising criterion and not the parents' comfort. In many jurisdictions the child has a legal right to know the identity of a parent, and in almost all of them the child's interests will be weighed in any later dispute.

Step 4 — Refer for legal advice, and say why. The referral question is specific: what are the client's obligations and rights in respect of maintenance, custody, registration and the child's knowledge? The therapist does not answer this. She does something more useful: she writes the question down for the client so that the client can put it to a lawyer, and she records that she did so.

Step 5 — Sequence the disclosure, if there is to be one. The order matters and there is no universally correct one. The options: mother tells the father, then both tell the child; the father tells the child without the mother; a family session with the therapist present; or deferral until the child reaches an agreed age. Each has costs. What the therapist can do is insist that the decision is made deliberately, with the child's interest stated aloud, rather than by drift.

Step 6 — Be present for the child's question. Children ask the questions adults avoid: "so who is my real father?" and "does he still love me?" The therapist's preparatory work with the parents is to write answers to those two questions in advance, in words a child can hear, and to agree that the child's relationship with the man who raised him will be described as a relationship and not as a technicality.

Where the therapist stops. At the door of the courtroom. Everything about maintenance, access, guardianship and registration belongs to the lawyer; everything about the child's experience, the parents' grief, the family's story and the disclosure conversation belongs to the therapist.

The vignette. Kola, 47, a school principal, attends alone. He says his wife has told him, in an argument, that their second child, now 14, is not his. He has said nothing to the child and has not slept in three weeks. He wants to know whether to ask for a test.

The therapist's first session does four things and none of them is advice. She stabilises — sleep, work, the suicidal ideation he mentions in passing and then retracts, which is assessed properly. She establishes what he knows: his wife's statement in anger, no test, no corroboration. She names the two questions that are currently fused in his mind and must be separated: is it true? and what do I do about my son? And she writes, for him, the questions for a lawyer: what is his legal position if the child is not his; what obligations continue; what rights does he have; what happens if he seeks a test without the mother's consent.

The clinical work over eight sessions is not about the test. It is about the fact that Kola has not been able to look at his son for three weeks, and that the boy has noticed. The most urgent task in the case is that a fourteen-year-old is being silently abandoned by a father who is waiting for a fact. Whatever the fact turns out to be, that is the harm that is happening now.

5.3 HIV and other communicable conditions

The clinical reality. A client discloses that he is living with HIV and has not told his wife. He is on treatment and virally suppressed. Or a client discloses that her husband is living with HIV and has not told her. Or a client discloses that he has been diagnosed with hepatitis and does not intend to tell his partner. In each case the therapist holds information whose non-disclosure exposes an identifiable person to a serious condition.

Step 1 — Know the rule before the case arrives. The therapist's practice manual should contain four answers, reviewed annually: (a) is there a statutory duty to disclose to a public health authority, and within what period? (b) is there a statutory duty or permission to disclose to an identifiable partner, and to whom? (c) is non-disclosure or exposure a criminal offence, and does the therapist's knowledge create any liability? (d) does the jurisdiction operate a partner-notification service, and how is it accessed? Four questions, four sentences. The therapist who cannot answer them is not competent to hold this case.

Step 2 — Assess the actual risk. The therapist needs, not rumour: the client's treatment status, whether viral suppression has been achieved and documented, whether protection is used, and what the partner knows. In many of these cases the risk is far lower than the client's fear, and in some it is higher than the therapist's first assumption. The risk assessment is not the therapist's opinion; it is what the client's physician has documented, and part of the therapist's work is to get the client to a physician.

Step 3 — Separate shame from risk. The commonest presentation is a client who is not avoiding disclosure because of danger but because of shame and the fear of being seen as dirty, unfaithful, or diseased. Naming this distinction changes the work: "I am not asking you to disclose because you have done something wrong. I am asking you because there is a person who could be infected and who is not being given the chance to protect herself."

Step 4 — Use the partner-notification route if it exists. Where a service exists, it can be used anonymously or with the client's involvement, and it frequently resolves the case: the partner is told by a trained worker, the client is spared the first sentence, and the therapist's dilemma dissolves. This route is under-used simply because therapists do not know it exists.

Step 5 — Plan the disclosure hour by hour. Where the client will disclose, the plan is specific: when — not "soon", but a date and a time when the house is empty; who — alone, or with a trusted person; what — the diagnosis, the treatment, the risk to the partner, and the offer to attend a clinic together; and what if — the two worst-case responses, each with a plan, including where the client sleeps that night.

Step 6 — Where the client refuses and the risk remains. Then the therapist states her position in terms of her own conduct, in writing, and seeks immediate supervision and legal advice if the jurisdiction imposes a duty. Three options, in order of preference: the client discloses within a stated period; the client authorises partner notification; or the therapist discloses what the law requires and tells the client in advance that she will. What the therapist may not do is continue as though nothing has happened, because in this case the continuation of silence has a named potential victim and a documented date.

The vignette. Mrs Ifeoma Nwosu, 38, a seamstress, attends for anxiety. In session four she discloses that her husband, Emeka, was diagnosed eight months ago, is not on treatment, has refused to tell her, and she learned of it from a pharmacy receipt. Her question is not whether to tell him she knows; it is whether to leave. The therapist's tasks here are inverted: the exposed person is the client, the secret's holder is the absent husband, and the clinical work includes her own testing, her own risk, her own safety and the decision about the marriage. The therapist does not contact the husband. She supports the client in her own disclosure if the client chooses it, assists her to a clinic, and records the reasoning. The case then moves into Week 9's territory, because a woman whose husband has concealed a diagnosis for eight months is a woman whose household may become unsafe when she says so.

5.3a The third party's spouse: the question with no clean answer

The question. The client's affair partner is married. Her husband does not know. Does the therapist do anything?

What is not available. The therapist cannot contact him. That would be a disclosure of her client's confidential information to a stranger, and it is prohibited regardless of the moral merits. Any answer that begins "the therapist should tell him" has misunderstood the ethics of the role.

What is available. Four options, in ascending order of intrusiveness, and the therapist should be able to name all four and say which she would press for and why:

1. **The client tells him.** The cleanest, rarest, and the one the therapist should raise first, explicitly and without embarrassment: "he is being deceived in the same way your wife was. Is that something you are willing to end?" Many clients have not considered it, having classified the affair partner's marriage as not their business.
2. **The client tells the affair partner that he will tell, or that she will, within a stated period.** A pressure that is applied to the right person.
3. **The couple decides together**, which happens more often than clinicians expect once the injured partner is past the acute phase: the betrayed spouse frequently wants the other household to know, and the decision is then a joint one that the therapist can support.
4. **Nobody tells him**, and the therapist records that the question was raised, what the couple decided, and why. This is a defensible outcome and it must be a decision rather than an omission.

The safety filter that overrides the order. Where the affair partner's husband is known to be violent, the disclosure may expose the affair partner and the client to serious harm. In that case the correct answer is frequently option 4 with a documented reasoning, and the therapist who presses for disclosure has imported a moral position into a risk calculation.

The therapist's own view. The therapist is entitled to have one and is not entitled to impose it. The workable practice is to state it once, briefly, as a question rather than a

verdict — "have you thought about what happens to his wife?" — and then to work with the answer.

5.4 Anatomy of the disclosure session

A structure for the session in which the affair is first discussed with both partners present. Ninety minutes, not fifty, and scheduled deliberately rather than arising inside a routine session.

Phase	Minutes	What the therapist does
Frame	0–8	Names the purpose; states the rules (no detail beyond the necessary; no leaving the room; a break available on request); reminds both that no decision about the marriage is being made today
The account	8–30	The involved partner gives the outline, using the five questions. The therapist controls the pace and stops the account when it moves into detail that serves no safety function
The questions	30–55	The injured partner asks; the therapist filters — "that is a detail; ask instead what you need to know to feel safe"
The response	55–70	The injured partner's reaction, uninterrupted. The therapist does not manage it, soften it, or interpret it
The commitments	70–85	Concrete, verifiable, dated: contact ended, method of proof, transparency arrangements, who else is told
The close	85–90	Both partners state one thing they will do before the next session; the therapist summarises in five sentences and records them

The three rules of the room. First, the involved partner answers; the therapist does not answer for him. Second, the injured partner's anger is not moderated in this session — it is the appropriate response and moderating it protects the wrong person. Third, the session ends with commitments rather than with feelings, because commitments are what the injured partner actually needs and feelings are what the involved partner finds easier to offer.

5.5 Case: the Okonkwos, second year

The household. Ada Okonkwo, 35, a pharmacist; Chuka Okonkwo, 40, a logistics manager; Ebuka, 10. They were the family in Week 1, where the clinic's policy on individual disclosures was explained, recorded and agreed.

The referral. Ada telephones on a Tuesday morning. "I found something on his phone. I need to see you before I do anything." The therapist offers a same-day individual appointment, states the policy again at the start of it, and Ada agrees to it a second time.

What Ada knows. A message thread on Chuka's phone, seen four days ago. A woman's name, Nkechi. Messages that are affectionate and explicit, spanning roughly seven months. Ada has photographed eleven messages, has told no one, and has not confronted him. She has not slept more than four hours in four nights and has twice been unable to drive because of shaking. She has begun looking up divorce lawyers on her phone and has not called one.

The assessment in that session.

- Acute stress: intrusive images, hypervigilance, sleep loss, appetite loss, intrusive checking of his phone at night. Not suicidal, but passive ideation on direct enquiry ("sometimes I think I should just disappear") which is assessed and planned for, and reviewed at every subsequent session.
- Risk: no history of physical violence. Chuka has never been violent; he is, in Ada's description, "the one who goes quiet". Ada's own anger is substantial and she is frightened of it.
- Disclosure history: the children know nothing. Ada's mother knows nothing. Ada's sister knows "something is wrong".
- Immediate functioning: she cannot concentrate at work and has taken two days of leave.

The therapist's four tasks in that first session. Stabilise (sleep, food, the name of a person she can call at night). Separate the questions (is it happening; is it over; what do I do; who do I tell — four questions, and they cannot be answered in one week). Establish what she needs to know (whether it is ongoing, whether protection is used, whether there is a pregnancy, whether money has moved — the safety questions, and no others). And set the frame for the next session: "you will not decide anything this week. You will come back on Thursday, and we will decide what to do about the conversation with him."

Session two, three days later — the question of the individual session. Ada wants to see the therapist alone again, indefinitely, "to decide what I am going to do". The therapist applies Week 3: the couple is the client; individual sessions are permitted and are not a parallel therapy; and the plan must be that the disclosure moves into the couple's room. Ada agrees, with one condition — she wants the disclosure to happen in the therapist's office, not at home, because "at home I will either scream or say nothing".

Session three — Chuka alone. He attends, having been told by Ada only that she has "seen something" and that they have an appointment. He is guarded, then, at minute nineteen, says: "I know why I am here."

What he discloses: a relationship of seven months with a colleague, Nkechi, who is also married and has two children. He says it ended "in a manner of speaking" three weeks ago. He has not told Ada. He says he intends to, "when things are calmer". He says his marriage has been "dead for two years". He asks, twice, whether the therapist will tell Ada.

The therapist's answers, in order. "I will not tell her what you have told me. And I am not willing to hold this while I sit with both of you — which means it needs to move, and the shape of that is what we will decide now." She does not accept "when things are calmer", because there is no calmer. She asks the safety questions: protection, pregnancy, money, and the affair partner's marriage. She asks what he has told Ada about the last seven months — the lies, specifically — because that is the material the disclosure session will have to address. And she says the sentence that determines the case: "you will tell her, in my room, on Thursday. I will help you prepare what you say. She has already seen the

messages, so this is not a revelation; it is the truth after four days of her not knowing whether she is going mad."

Session four — the disclosure session, ninety minutes. Both present. Ada speaks first, for six minutes, without interruption: what she saw, what the four days have been like, and the sentence she has rehearsed — "what I need from today is not an explanation, it is the truth." Chuka then answers the five questions. He gives the outline: seven months, a colleague, twice-weekly, ending three weeks ago — although Ada's messages suggest contact nine days ago, which he then corrects. He names the lies: the late nights, the "team retreat", the weekend he said he was with his brother. He states what he has done: message sent ending it, contact blocked, and an agreement to move teams at work next month. He states what he has not done: he has not told Nkechi's husband. And he answers Ada's question, "why", with the sentence that becomes the centre of the treatment: "because I did not want to be the man who had failed, so I stopped telling you anything."

The detail question, in the session. Ada asks, at minute forty, "was she better than me?" The therapist intervenes, exactly as the protocol requires: "that is a detail, and I am going to stop it — not to protect him, but because you will carry the answer for years and it will not tell you anything you need. What you need to know is whether he wants to be married to you, and that is a question he has to answer." Ada accepts this, and later says it was the most useful sentence in the session.

The commitments at minute seventy. Four, all dated. Chuka sends Ada a written account of the timeline by Saturday. He books and attends an STI screen within ten days and shows her the result. He agrees to leave his phone in the kitchen at night for three months. He agrees to a joint session every week for eight weeks. And on the fourth point — telling Nkechi's husband — he agrees to decide by the following Thursday, and the therapist records the question as open.

The next eight sessions. The treatment is not about the affair. It is about the two years in which Chuka stopped telling Ada anything, and about his belief that a man who admits failure loses his household, which is his father's belief and which he has never examined. Ada's work is separate and harder: she has to decide whether she can live with a man who lied competently for seven months, and the therapist's job is to keep that question open rather than to sell the marriage. In session eleven Ada says, "I do not trust him. I have decided to see whether trust comes back." That sentence, not the affair, is the case's turning point.

What the case teaches.

1. **The individual session was the instrument, not the problem.** Without three individual sessions — Ada's two and Chuka's one — there would have been no prepared disclosure, and the conversation at home would have been the screaming match Ada predicted.
2. **The policy was stated in Week 1 and applied in Week 5.** The therapist could hold Chuka to a disclosure because the clinic's policy had been explained to both of them a

year earlier and recorded. A therapist who improvised the policy on the day would have been negotiating from nothing.

3. **The safety questions are asked of both partners, separately.** Ada was asked about violence, ideation and functioning; Chuka was asked about protection, pregnancy, money and the affair partner's marriage. Neither set of questions would have been answerable with both in the room.
4. **The disclosure session was structured, and the structure was the therapy.** Frame, account, questions, response, commitments, close: six phases and ninety minutes, and none of it was improvised.
5. **The treatment's subject was not the affair.** The affair was the event; the marriage's two-year silence was the case. Finding that distinction is the clinical work of session three.

5.6 Transcript: the refusal of "when things are calmer"

Session three, Chuka alone, minutes twenty-two to forty. Annotated.

> **C1 Chuka:** I will tell her. When things are calmer. She is not in a state to hear it now. **C2 Therapist:** (pause) There is no calmer. — Four words, and the refusal to accept the most common postponement in this literature. "When things are calmer" is not a plan; it is a description of the client's hope that the problem will become smaller on its own. **C3 Chuka:** You do not know her. She will — (stops) **C4 Therapist:** I do know her a little. I saw her on Tuesday. (pause) She has not slept in four nights and she has been photographing your phone. She is already in it, Chuka. The only question is whether she is in it alone. — The move that ends the postponement: not an argument about duty, but a description of what the delay is actually costing the person the client says he is protecting. **C5 Chuka:** (long pause) I did not think of it like that. **C6 Therapist:** Most people do not. (pause) Tell me about the last seven months. Not the relationship — the things you said to her. — The pivot from the affair to the deception. This is the technical heart of the session: the affair is what happened; the lies are what the disclosure must address. **C7 Chuka:** I said I was working late. I said the retreat was in Abuja. I said — (pause) — I said I was tired. That one was true. **C8 Therapist:** So there was a true thing in it. — A reflection that does something precise: it neither excuses nor condemns, and it registers that the client's marriage was not simply a lie. **C9 Chuka:** (quickly) It is not her fault. Ada. It is not her fault. **C10 Therapist:** I did not say it was. (pause) But you have just told me you were afraid of being the man who failed. Where does that come from? — The question that opens the third session's real material, asked without any suggestion that it excuses the affair. Note that the therapist first refuses the implication (C10, first sentence) and then asks the question, in that order — reversing the order would have sounded like an excuse. **C11 Chuka:** My father. He was — he lost his job when I was twelve, and my mother's brothers took over the house. I watched him become nothing. In our place, a man who cannot provide is — he is not a man. **C12 Therapist:** (pause) And in seven months, did you become him? **C13 Chuka:** (long pause) Every time I came home. — The sentence the case will be built on, arrived at in session three, without the therapist having proposed it. **C14 Therapist:** Then we have something to work with. (pause) On Thursday you will tell Ada the truth about the seven months, and I will be in the room, and this — what you have just said — is what we will be working on afterwards. — The two-part close: the disclosure is not negotiable, and the treatment is promised. Both parts are necessary; either alone fails.

What the transcript shows. The refusal of the postponement took eleven words and no moral argument. The relevant contrast is with the therapist who says "you have an obligation to tell her", which invites a debate about obligations; the therapist here says what the delay is doing, which cannot be debated because it is observable.

5.7 Laboratory: three secrets, three protocols

Aim. To produce, in ninety minutes, three one-page protocols — infidelity, paternity, HIV — for a named practice, each with the clinical sequence, the referral trigger, the documentation and the jurisdictional question.

Procedure. Groups of four, each group assigned one secret, twenty minutes to draft, then ten minutes to present to the other groups, who must find the missing element. Each group must also write the sentence it would use to explain the limit of confidentiality to a client in that case.

Marks (20).

Criterion	0–1	2–3	4–5
Sequence	No order	Order present but not justified	Order justified by exposure and by the client's capacity
Referral trigger	None	Vague ("refer if needed")	A specific question for a specific professional, written out
Jurisdiction	Not addressed	Mentioned	The four questions named, with a source and a review date
Documentation	Nothing	Narrative	Classification, reasoning, decision, date, supervision
The sentence	Moralising or evasive	Accurate but cold	Accurate, brief, and delivered without blame

The commonest failure. Groups write "the therapist should advise the client to disclose". That is advice about a third party's conduct, it is outside the role, and it is marked in the lowest band however sound the underlying ethics.

5.8 Six errors

Error 1 — Adjudicating the affair. "What he did was wrong" is true, and said in session one it makes the therapist the injured partner's ally and the involved partner's judge — and the involved partner, who is necessary to the repair, stops speaking.

Error 2 — Answering the detail questions. The injured partner asks, the therapist permits, and both carry images for years. The filter — facts necessary for safety and choice — must be applied by the therapist, in advance, out loud.

Error 3 — Treating paternity as a therapy question. The therapist who opines on maintenance, custody or registration has left her competence and possibly her indemnity. The referral is written down, with the question specified.

Error 4 — Assuming the local HIV rule from memory. The rules differ, they change, and the consequences of being wrong are criminal. Four questions in the practice manual, reviewed annually.

Error 5 — Continuing a treatment that is being conducted around an undisclosed exposure. The therapist who sees the couple weekly while the husband's diagnosis remains hidden from the wife is not neutral; she is a party to the exposure.

Error 6 — Deciding the third-party question by omission. The affair partner's spouse is either told, or the question is raised and recorded as decided against. What is not permissible is never raising it.

5.9 Culture, faith and the three secrets

Infidelity and the shape of the household. In a polygynous or extended household, the definition of infidelity is not the therapist's to set. A second relationship may be adultery to one wife, a breach of an agreed arrangement to another, and a family matter to be resolved by elders to a third. The therapist's first task is to ask what the couple's own understanding was — what was promised, to whom, and in front of whom — because the injury is measured against the promise and not against the therapist's model of marriage.

The elders' jurisdiction. Where a family council has authority, the couple may want the matter resolved there rather than in the therapy room, and the therapist may be asked to attend or to report. Attending a family council is sometimes possible and requires a clear contract about confidentiality; reporting to it almost never is. The workable answer is that the therapist will help the couple prepare what they will say to the elders.

Paternity and lineage. In households where lineage carries inheritance, naming and belonging, the disclosure of non-paternity is not only a relational event; it can alter a child's standing in a family and a community. This raises the stakes and does not change the criterion — the child's welfare — but it changes the timing, the audience and the planning. The therapist who treats it as a Western-style family secret has underestimated the case.

HIV, blame and the church. Where a diagnosis is read theologically — as judgement, as a test of faith, or as a punishment on a household — disclosure carries a second injury. Congregational responses vary from extraordinary care to exclusion, and the client knows which is likely in her own congregation better than the therapist does. Asking "if this became known in your church, what would happen?" produces information that belongs in the risk assessment.

The communitarian objection. "In our culture, family matters are not discussed outside." This is true and it is not an answer to the exposure question. The therapist's response separates the two: "I agree that this is family business. The question I am asking is not whether to keep it in the family — it is whether the person who is being exposed is inside the information or outside it."

> **DEBATE**
> "When a client refuses to disclose an HIV-positive status to a partner, the therapist's duty to the partner overrides confidentiality." Argue for: an identifiable person is at risk, the client's refusal converts the therapist into a participant, and public health law in many jurisdictions supports disclosure. Argue against: disclosure by the therapist destroys the confidence on which testing and treatment depend, the partner-notification route exists precisely to avoid it, and the therapist's duty is defined by statute and not by the therapist's conscience. Then state the rule you would work under and the jurisdiction you would work in.

5.9a The referral note to a lawyer

Therapists refer to lawyers constantly and badly. A referral that says "client needs legal advice" produces a lawyer who cannot begin. A referral that specifies the question produces advice the client can use.

The four questions worth writing down. First: what is the client's legal position in respect of the children if the facts are as stated? Second: what must the client do, or refrain from doing, in the next thirty days to avoid prejudicing that position? Third: what records exist, and can any of them be compelled? Fourth: what does the client need to know about the timing of any disclosure?

What the therapist does not put in the note. Her clinical opinion of the parties, her view of who is at fault, and any speculation about facts the client has not established. The note is a question, not a case formulation, and the lawyer's file may become a document in the proceedings.

What the therapist records afterwards. That the referral was made, on what date, with what question, and that the client was advised to obtain advice before making any decision with legal consequences. Six lines. They are the difference between a therapist who acted within her competence and one who did not.

5.10 Evidence note

What the research supports. Affair recovery research describes a reasonably consistent trajectory of acute stress, meaning-making and rebuild-or-end, with the injured partner's intrusive symptoms typically prominent for weeks and the couple's trust rebuilt through consistent verifiable behaviour rather than through reassurance. Disclosure of HIV status is associated with better treatment engagement and lower transmission risk, and partner-notification services increase disclosure rates; the public health literature is clear that supporting voluntary disclosure outperforms mandatory approaches on most measures. The research on secrecy in families shows that maintained family secrets are associated with poorer wellbeing in the person holding them, and that disclosure is most damaging when it is discovered rather than told.

What is weaker. There are no trials determining the optimal content of an affair disclosure, and the clinical debate between full disclosure and a facts-only approach is not settled empirically. Paternity disclosure has a small qualitative literature and almost no outcome data, largely because the situations are heterogeneous.

What is not supported. That a couple cannot recover from an affair — the outcome data on couple therapy following infidelity is modest but positive. That disclosure of HIV status always improves relationships — it can precipitate violence, which is why safety planning precedes disclosure. And that the therapist's judgement about the moral seriousness of the secret should determine the urgency; the exposure assessment does that.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Seven months of an affair discovered by a phone message, and seven months disclosed by a client in session twenty, are the same fact with two different prognoses.

5.11 Key terms

Betrayal trauma — the traumatic stress response to the discovery of a significant relational betrayal.

Disclosure scene — the first structured conversation in which the affair is discussed with both partners.

Exposure assessment — the identification of who is put at risk by the continuation of a silence.

Presumption of paternity — the legal presumption that a child born within a marriage is the husband's; jurisdiction-specific.

Partner notification — the public-health route by which a partner is informed of exposure, anonymously or with the index client's involvement.

Viral suppression — the reduction of viral load to undetectable levels; relevant to risk assessment and not a substitute for disclosure.

Safety filter — the rule that where disclosure would create a risk of violence, the risk governs the plan.

Third-party claim — the moral (not legal) claim of a non-client on information that affects them.

5.12 Self-test

1. State the common structure of the three secrets in this week.
2. What are the four questions the practice manual must answer on communicable disease?
3. Which detail questions may the therapist permit, and which must she stop?
4. Why is "when things are calmer" not a plan?
5. Name the four options for the affair partner's spouse, and the filter that overrides them.
6. What does the therapist do when paternity is in question and the client asks for advice about custody?
7. Give the six phases of the disclosure session with their timings.
8. In the Okonkwo case, what was the treatment actually about?

Answers. (1) An established fact, a person who does not know it, and a third party exposed by the continuing silence. (2) Whether there is a duty to a public health authority and within what period; whether there is a duty or permission to disclose to an identifiable partner; whether non-disclosure or exposure is an offence; and whether a partner-notification service exists and how it is accessed. (3) Permit facts necessary for safety and choice — duration, protection, whether contact continues, whether the affair partner's household is affected; stop questions about sexual practice, comparison, and the other person's appearance. (4) Because it names a state of affairs that will not arrive on its own and does not specify who does what by when; the delay is itself the harm. (5) The

client tells him; the client tells the affair partner that he will tell; the couple decides together; or nobody tells him and the reasoning is recorded. The safety filter — a risk of violence — overrides all four. (6) She says she does not give legal advice, writes the client's question down for a lawyer, and records that she did so. (7) Frame 0–8; account 8–30; questions 30–55; response 55–70; commitments 70–85; close 85–90. (8) The two years in which Chuka stopped telling Ada anything, and his belief that a man who admits failure loses his household.

5.13 Reflective task

Write three hundred words on the following: a couple arrives having discovered an affair, and you find yourself hoping they will separate. What in your own history is producing that hope, and how would you know if it had begun to shape your questions? Do not resolve the question. Locate it.

5.14 Further reading

- Glass, S. on the anatomy of affairs and on Not "Just Friends" — the standard clinical account of boundaries and affair types.
- Gottman, J. on trust and betrayal in couples — the framework for the trust-rebuilding tasks.
- Perel, E. on the meaning of affairs — the counterweight to purely behavioural accounts, useful for the third session's work.
- The WHO guidance on HIV partner notification and the UNAIDS materials on disclosure — the source for the pathway in section 5.3.
- Any current local statute or professional guidance on communicable disease disclosure, read in the original and filed in the practice manual.
- A family law primer for your jurisdiction, for the reference list and not for advice.

Next: Week 6 — Boundaries and multiple relationships.

Week 6 — Boundaries and multiple relationships

Learning outcomes. By the end of this week the student can (1) distinguish a boundary crossing from a boundary violation and explain why the difference is the whole of the topic; (2) apply the four tests to any multiple relationship and reach a defensible decision; (3) manage the unavoidable dual relationships of small- community practice without either pretending they do not exist or abandoning the client; (4) state and defend a personal rule for touch, self-disclosure, gifts, bartering and digital contact; and (5) recognise and manage sexual attraction to a client as a professional event rather than a personal failing.

The distinction the week is built on

A **boundary crossing** is a departure from the usual frame that is made for the client's benefit, is discussed, and is reversible. A **boundary violation** is a departure that serves the therapist, is concealed, and often is not reversible. The two look similar in a textbook and are entirely different in a file.

	Crossing	Violation
Whose need	The client's	The therapist's
Disclosed	To the client, to supervision, to the record	Concealed, or minimised
Reversible	Yes — the frame can be restored	Rarely; the relationship is altered
Example	A fifteen-minute extension in a crisis, noted and discussed	Sessions that routinely run long because the therapist enjoys them
Example	Attending a former client's wedding after discharge, with the frame discussed	Attending a current client's family event as a guest
Example	Accepting a bowl of fruit at the end of a course of treatment	Accepting a car

The pedagogy follows from this: students who are taught boundaries as prohibitions become anxious and rigid, and rigid therapists fail their clients at exactly the moments when flexibility is clinical. Students who are taught the crossing/violation distinction can think, and thinking is what the situation will require.

Why this topic is examined so heavily in the codes. Because boundary violations are the commonest route by which therapists harm clients, and because the sequence is well documented: a gradual erosion, always initiated by small kindnesses, always rationalised as therapeutic, and almost always preceded by isolation from supervision. The literature on therapist sexual misconduct finds that the great majority of cases were preceded by non-sexual boundary crossings over months, and that colleagues often noticed and said nothing.

The error that costs most in this subject. Teaching the topic as though it were about sex. It is about power, and the sexual cases are the endpoint of a sequence that begins with the therapist becoming special.

6.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	Crossing and violation: five paired examples, sorted by the class before any theory	Live sorting
10–22	Five concepts: power, role conflict, the slope, the small community, the felt sense	Lecture, board diagram
22–40	The four tests and the decision matrix, applied to eight scenarios	Small groups, then plenary votes
40–55	The unavoidable dual relationship: the community contract	Demonstration of the opening conversation
55–70	Touch, gifts, barter, home visits, digital contact: the rule and the rationale	Lecture with cases
70–80	Attraction: recognition, disclosure to supervision, and the decision tree	Short lecture; written anonymous question

Minutes	Content	Teaching form
80–90	The Alabi case	Case presentation, students decide

6.2 Five concepts that organise the week

Concept 1 — Boundaries exist because the relationship is asymmetric. The client tells the therapist what she has told no one; the therapist tells the client almost nothing. The client depends on the therapist's judgement; the therapist depends on the client for a fee. That asymmetry is not an unfortunate feature of therapy; it is the mechanism by which therapy works, and every boundary in this week's material exists to protect it. A boundary that is justified by "professionalism" in the abstract will not survive pressure. A boundary justified by the asymmetry will, because the therapist can see what it protects.

Concept 2 — Role conflict degrades judgement before it degrades conduct. The therapist who is also the client's landlord does not begin by acting badly; she begins by thinking differently. She notices that she is reluctant to challenge the client about the rent, or that she is unusually warm about the client's financial difficulty. The judgement goes first. This is why role conflicts are examined at the point they are created and not at the point they produce harm.

Concept 3 — The slope is real, and it is walked one small step at a time. No therapist decides to have a sexual relationship with a client. A therapist decides to take one phone call, then to meet for coffee "to discuss the crisis", then to accept a lift, and at each point the previous step has made the next one seem continuous. The clinical use of the concept is not to frighten students but to give them a rule: the sequence is interrupted at the first crossing, when it is cheap, and the interruption is a conversation rather than an act of will.

Concept 4 — In small communities, multiple relationships are not avoidable; they are manageable. In a city of eight million a therapist can decline a case and the client will find someone else. In a town of thirty thousand with two trained therapists, refusing the mayor's family can mean the family receives no service. The ethical question therefore shifts from may I avoid this? to how do I manage this honestly? — which is a harder question and the one this week spends most of its time on.

Concept 5 — "It felt right" is not a test. The felt sense is a poor guide in this domain because the therapist's warmth, the client's gratitude and the pleasure of being needed all feel like evidence of a good relationship. The test is external: whose need is served, what would supervision say, what would the client say in five years, and is it in the record?

6.2b The five-minute boundary decision

The four tests produce a decision, and in practice the therapist has five minutes and no matrix to hand. The compressed version, used in the room, is four questions asked in this order.

Question one — what would I have to stop doing? The concrete loss is the fastest indicator of a role conflict. A therapist who realises that she could no longer challenge the client about money has found the conflict without needing a theory.

Question two — who would know first? If the answer is "nobody", the arrangement has already failed. Every defensible dual relationship in this chapter was declared before it began; every indefensible one was discovered.

Question three — can the client say no? A favour, an offer of employment, a discount: each is a proposal made by a person the client depends on. If the client cannot refuse without cost, the arrangement is exploitation whatever its intention.

Question four — what is my sentence? The therapist must be able to say, in two sentences and without embarrassment, what the arrangement is and why. A clinician who cannot construct this sentence has not decided anything; she has drifted, and drift is the mechanism of the whole chapter.

What to do with the answer in the five minutes. Three outcomes only. Proceed with the contract write-up. Decline with the four-part script. Or say "I need to think about this and I will tell you on Thursday", which is always available, is never wrong, and is the answer most often forgotten under pressure.

The one arrangement that cannot be decided in five minutes. Any proposal that involves money, employment, or a sexual or romantic dimension — including a proposal that arrives from the client, obliquely, in a sentence that could be read two ways. Those go to supervision before any answer is given, and the client is told that the therapist is thinking about it, which is true and which is not a rejection.

6.2a The four tests, and the decision they produce

The four tests from the scaffold are the right ones. What follows is how to use them so that they produce a decision rather than a discussion.

1. **Objectivity.** Could the second relationship impair my judgement about this client's treatment? Not "will it" — could it. The answer is almost always yes in some degree, and the useful question is what specifically would change.
2. **Exploitation.** Could the second relationship exploit the power difference? A loan to a client, a business partnership, a favour that the client cannot refuse — each is exploitation even where the therapist's motive is generous.
3. **Foresight.** Would the client experience this as exploitative later? This is the test that is easiest to answer dishonestly and most useful when answered in the client's voice. "In three years, telling a friend about her therapy, how would she describe this?"
4. **Avoidability.** Is the second relationship avoidable without depriving the client of the service? A gift from a bereaved family is not really avoidable; a business partnership is.

The decision matrix.

Objectivity impaired?	Exploitation possible?	Avoidable?	Decision
No	No	Yes	Decline, briefly and without a lecture
Slightly	No	Yes	Decline; discuss in supervision
Yes	No	No	Proceed with a written contract and supervision review
Yes	Yes	No	Proceed only with a formal risk management plan, documented, reviewed monthly
Yes	Yes	Yes	Do not proceed. Transfer the case

The three columns that matter. Written contract: the conversation with the client about how the two roles will be handled, recorded in the file in the client's presence and with the client's agreement. Supervision review: a named supervisor who knows about the arrangement and a date at which it is reconsidered. Transfer: the option that therapists resist and that is frequently correct.

The sentence for declining. "I am not able to take this on, and I want to tell you why so that it does not feel like a rejection: I know your family socially, and that would make it hard for me to be a good therapist to you. I can give you two names." Brief, honest, and it does not invite negotiation. The therapist who explains at length has made the client responsible for accepting the refusal.

6.3 The unavoidable multiple relationship: the community contract

Where the therapist and client will inevitably meet in another role — the same church, the same school, a shared extended family, a village — the work is not avoidance but an explicit contract, made at the start, recorded, and revisited.

The four elements of the contract.

1. **The naming.** "We are going to see each other at church, and at your sister's shop, and probably at the market. I want us to agree now how we will handle that, so that neither of us is caught out."
2. **The greeting rule.** Agreed in advance: the therapist will greet the client normally and will not refer to therapy; the client may decide whether to acknowledge the therapist first; and the therapist will follow the client's lead in public. This small rule prevents the commonest injury, which is a client left standing in a market while her therapist pretends not to see her.
3. **The information rule.** What the therapist will do with information acquired in the community role. "If your mother tells me at church that you have lost your job, I will not raise it in session as though you had told me. I will wait for you to bring it, or I will ask whether you want to talk about it." This is the single most useful clause in the contract and almost never written.
4. **The review.** A date at which the contract is looked at again, because the community roles change — the client's daughter joins the therapist's son's class, the client is elected to the church committee that employs the therapist's husband.

What the contract does not solve. It does not remove the judgement problem. A therapist who is a deacon and a therapist in the same congregation has two sources of authority over a client, and the contract manages the boundary without dissolving the role conflict. Where the conflict is structural — the therapist holds a leadership role in a community the client belongs to — referral is the better answer, and saying so is not a failure.

6.3a The role conflicts that recur

The client who is a colleague. A fellow therapist, a nurse, a teacher in the same school. The specific hazards: the colleague knows the therapist's reputation, may be supervisee or supervisor, and will meet the therapist professionally after treatment ends. The management: an explicit discussion of what will happen at professional meetings, no treatment of the colleague by a person who has authority over them, and no use of the colleague's clinical knowledge in the treatment.

The client who is a student. Power is at its maximum and cannot be contracted away. A therapist who teaches the client, examines the client, or supervises the client's placement should not also treat them, and the correct answer is referral. Where a training clinic has no alternative, the treatment is conducted by someone with no assessment authority over the student, and this is written down.

The client referred by a friend. The friend will ask how it is going. The therapist's answer, agreed in advance with the client: "she is attending and I am not able to discuss her." Said once to the friend, in the first week, without apology. The alternative — "it is going well" — is a disclosure.

The client whose family employs the therapist's spouse. A genuine conflict of interest in small economies. The test: would the therapist be able to challenge the client's behaviour if the client's family were paying her household? Where the answer is no, transfer.

The former client who becomes a friend. The codes differ, and the safest practice is that the relationship does not convert, or converts only after a substantial interval and with the recognition that the client may return to treatment. A former client who is now a friend cannot come back as a client, and the therapist should say so at the point the friendship is contemplated rather than at the point it is needed.

The client who becomes famous, or whose case becomes public. The therapist's obligation not to use the material does not lapse when the case is in the newspapers, and the temptation is at its maximum precisely then.

6.3b The referral conversation, scripted

Declining is a technique, and like any technique it improves with rehearsal. The four-part script below takes about two minutes and leaves the client with a service rather than a refusal.

Part one — the decision, stated first. "I have thought about this and I am not going to be able to take the work on." First, because a client who hears the explanation first spends it looking for the loophole.

Part two — the reason, in terms of the client's interest. "The reason is that I know your family outside this room. If I were your therapist I would already know things about your husband, and you would be watching what you said to me. That would make the therapy worse, not better."

Part three — the alternatives, specific. Two names, with a sentence about each, and an offer to telephone ahead: "Dr Bello works with couples and has a clinic on Tuesdays; Mrs Ojo is closer to you and works evenings. I can call either of them today and tell them that you will be in touch, without telling them anything about you."

Part four — the door. "If neither works, come back to me and I will find you a third name. What I will not do is leave you with nothing."

What the client usually says, and the answer. "Is it because of something I did?" — "No. It is about me and where I worship, and nothing about you." "You are the only one who understands." — "I have met you twice; what you are responding to is that somebody listened. Someone else will listen, and they will be able to hear more than I can."

6.4 Touch, gifts, self-disclosure, barter, home visits and the telephone

Each of these is sometimes therapeutic and always requires a rationale that can be spoken aloud in supervision. The rule for each is stated here as a personal rule, because a therapist who has not pre-decided will decide inside the moment, and the moment is designed to make the generous option feel right.

Touch. The rule: touch is used only where it is a recognised part of the therapeutic method being practised, or where it is the ordinary human response to acute distress and is brief and public. A hand on the shoulder of a woman who has just been told her husband is dying, in a room with a window, is not a boundary violation. A twenty-minute embrace at the end of every session is. The rationale: touch cannot be retracted, is interpreted through the client's history — particularly where the client has been abused — and is the commonest first step in the documented sequence toward sexual misconduct.

Gifts. The rule: small, culturally appropriate, given at a transition (the end of treatment, a bereavement, a birth), accepted with thanks and recorded. Anything of substantial value, anything given early in treatment, and anything that creates an obligation is declined with an explanation that protects the giver. The test: is the gift a communication or a transaction? In this region, food and cloth carry meaning and refusing them can be an insult; the therapist who declines a bowl of fruit to prove a point has damaged an alliance for an abstraction. The recorded note is short: "client brought fruit at the end of the eighth session; accepted, meaning discussed, recorded."

Self-disclosure. The rule: brief, about the therapist's experience rather than the client's, serving the client's work, and never requiring the client to care for the therapist. The test: if the client responded by consoling the therapist, would the disclosure have been appropriate? If not, it should not be made.

Barter. The rule: almost never. Where a client has no money and the alternative is no treatment, a barter arrangement may be ethical, and it requires a written value, a stated duration, and a supervision review. The specific hazard is that the client's service is now a clinical topic — the therapist who accepts tailoring from a client will have opinions about the tailoring.

Home visits. The rule: a home visit is a clinical intervention with a purpose, a time limit, and a documented rationale. Family therapy occasionally requires it: a home reveals what a clinic cannot. The visit is arranged in advance, attended with a plan, and recorded, and the therapist does not accept hospitality beyond water, does not stay for a meal unless the meal is the intervention, and does not go alone where safety is uncertain.

The telephone and the message. The rule: the channel is specified at the start, with its limits — "you may call between nine and six; if it is urgent and I am not available, go to the emergency number; I do not respond to messages after nine at night." The rule exists because the alternative is a therapist who is available at all hours to a client who has no other support, which is not a boundary the therapist can sustain and not a service the client can rely on.

Digital contact. The rule: no personal social media contact with current clients; no searching for a client online without a clinical reason and a record of it; no clinical material over unencrypted channels; and an explicit statement in the first session of what the therapist will and will not do online. The specific hazard of the photograph: a client's social media reveals information the therapist did not obtain clinically and cannot forget. Where the therapist has seen it, the fact is recorded, because the alternative is a treatment silently shaped by material the client never gave.

6.4a The boundary log

A one-page document, kept by the therapist, reviewed monthly in supervision. Five columns: date; the crossing; whose need it served; what was said about it to the client and to the supervisor; and whether the frame was restored.

Why a log rather than a memory. Because crossings are forgettable and their accumulation is the phenomenon worth noticing. A therapist who can see eleven small extensions over four months is looking at a pattern that no single session reveals, and the pattern is the early part of the slope.

What belongs in it. The session that ran twenty minutes over because a client disclosed a bereavement. The gift that was accepted. The lift home given in the rain. The client's child's school fee that the therapist "lent". The text message answered at eleven at night. Each is small, and the log's purpose is not to punish any of them.

What the review asks. Three questions. Is the same client appearing repeatedly? Is the same kind of crossing recurring with different clients, which points at the therapist rather than the case? And has anything been crossed that was not told to supervision, which is the only entry that is a violation rather than a crossing?

6.5 Sexual boundaries, and what to do with attraction

The absolute. Sexual contact with a current client is prohibited in every code this book draws on and is a criminal offence in many jurisdictions. There is no clinical argument, no cultural exception, and no case in which it is defensible. Sexual contact with a former client is prohibited by most codes for a stated period — commonly two years, sometimes five, sometimes permanently — and the period is a minimum rather than a permission.

Why the prohibition is absolute. Because consent is compromised by the asymmetry, and because the harm is severe and durable. Clients who have been sexually involved with a therapist show outcomes comparable to those of survivors of sexual assault by someone trusted: betrayal, confusion about their own responsibility, damage to their capacity to trust a helper, and in a substantial proportion, a subsequent suicide attempt. This is the single most harmful thing a therapist can do, and the research on it is not ambiguous.

Attraction is not the problem. Most therapists will feel drawn to a client at some point in a career, and the feeling is neither a symptom of impairment nor a predictor of misconduct. The problem is silence about it. The documented pathway to violation runs through secrecy: the therapist who cannot say "I am aware of feeling drawn to this client" to a supervisor is the therapist at risk.

What to do, concretely, in the week it happens.

1. **Name it to yourself** and write one line in a private note. The act of writing reduces the intensity and creates a record that can be discussed.
2. **Tell supervision**, in the same words: "I am aware of attraction to this client and I want to talk about how it is affecting the work." Supervisors should make this easy by asking routinely, because the first time it is said should not be the first time it is possible.
3. **Increase structure, not distance.** Do not abruptly become cold, which the client experiences as unexplained rejection and which often produces the pursuit the therapist fears. Keep the frame, keep the notes, keep the seating, and keep the session ending on time.
4. **Examine the material.** Attraction frequently carries information: the client resembles someone; the client is providing the admiration the therapist's life is not; the case is a repetition of an earlier one. The information is clinically useful and belongs to the therapist's own work, not to the client.
5. **Refer, if the feeling is not manageable.** Not as a punishment. The sentence: "I think you would be better served by a colleague, and I want to tell you that this is about my capacity and not about you."

Where a client declares love. The response is kind, brief, unambiguous, and does not interpret the client's feeling as pathology. "I am not able to be that for you, and I am not going to pretend I do not understand what you are asking. What I can be is your therapist, and I think that is worth more to you than the other thing." Then the clinical work: what the declaration is about, which is usually loneliness, gratitude, or a history in which care and romance have been the same thing.

6.5a What the codes actually say, and where they differ

The codes converge on the absolute prohibition and diverge on almost everything else, which is why a therapist moving between jurisdictions must read the version that governs her.

On sexual contact with current clients. Universal prohibition, and in a growing number of jurisdictions a criminal matter rather than a professional one. There is no divergence to report.

On sexual contact with former clients. The divergence is real: some codes impose a fixed interval — two years, sometimes five — others require the therapist to demonstrate that the contact is not exploitative, and some prohibit it permanently where the treatment was long or the client vulnerable. A therapist in a jurisdiction with a two-year rule who treats the interval as permission has misunderstood the rule, which is a minimum evidentiary threshold and never a warrant.

On non-sexual multiple relationships. The codes permit them where they are not exploitative and do not impair judgement or risk harm, and they require the therapist to document the reasoning. What none of them supplies is a list, and the absence is deliberate: the categories are local.

On gifts. Most codes say nothing explicit and the professional bodies' guidance varies from "small and culturally appropriate" to a value threshold in local currency. The therapist should know her own body's wording because a complaint will be assessed against it.

On barter. Permitted in some codes where the arrangement is not exploitative, is entered at the client's initiative, and is documented; discouraged in others.

On social media. The most recent guidance and the least settled. The consistent threads: no personal accounts for clinical use, no searching without clinical justification, and a written statement to the client at intake.

On touch. Almost entirely absent, which is why the reasoning in section 6.4 has to be constructed from the general principles rather than looked up.

6.6 Case: the Alabis, and the deacon's wife

The setting. A private practice in a state capital, and a church of about three hundred members where the therapist has worshipped for eleven years, sings in the choir, and is known to almost everyone as "the counsellor".

The referral. Mrs Yetunde Alabi, 44, a school administrator, is referred by her physician for depression following a miscarriage at fourteen weeks. Her husband, Deacon Samuel Alabi, 49, is a church officer and a friend of the therapist's husband. The couple have three children, 17, 14 and 9. The referral arrives by telephone from the physician, who does not know the therapist's church.

The first decision, before the first session. The therapist recognises the name and applies the four tests. Objectivity: impaired — she knows Samuel socially, has opinions about him, and has heard him preach. Exploitation: possible — she holds lay religious

standing in a congregation where the family worships. Avoidability: yes, in principle; there are four other therapists within reasonable distance. Foresight: the client, in three years, describing this therapy to a friend, would say "she is in my church". The matrix says: decline or transfer.

What actually happened, and why it is instructive. The therapist telephoned the physician and said she could not take the referral, giving the reason in one sentence. The physician asked whether that was not excessive: the family had asked for her by name. The therapist said it was not excessive and gave two names. The family, through Samuel, telephoned the therapist directly three days later and asked her to reconsider — "we would not be comfortable with a stranger; you are one of us."

The therapist's decision at that point is the pedagogically interesting one. She did not simply repeat the refusal. She offered a **consultation**: one meeting, with both of them, for the sole purpose of explaining the conflict, giving the two referrals, and — this is the part that matters — asking whether they wanted her to speak to the named therapist in advance so that the family would not have to begin from nothing. She did not take the case.

Why the refusal was right, in four sentences. Because the client's miscarriage, her grief and whatever is turning in her marriage will include material about her husband that she must be able to say to someone who is not in his network. Because the therapist would meet Samuel at a men's fellowship on Saturday and would be carrying his wife's account of him. Because a church of three hundred people cannot keep a secret, and the therapist's own presence in the congregation would make the client's attendance a piece of church news. And because the standing the therapist holds in that church is a form of authority the client cannot decline.

The alternative that was considered and rejected. The therapist considered taking the case with a community contract — the four elements of section 6.3 — and rejected it, for one reason: the contract manages the encounters and cannot manage the information. In a congregation, Samuel's friends will discuss his wife's depression with him, and the therapist will be present when they do.

The consultation, in practice. Fifty minutes, both present. The therapist said four things: that she was not going to be the therapist and that the reason was the church, not the family; that she was sorry, because she knew the refusal felt like a rejection and it was not one; that she would give two names and would speak to either of them if the family wished; and that she would do one more thing, which was to be clear that she would never discuss the family's business with anyone at church, including her own husband. The last sentence did more clinical work than the referrals.

The follow-up that makes the case worth teaching. Six weeks later, at a church event, the therapist met Yetunde. Yetunde said, in front of two other women, "I am seeing Dr Bello now. It is going well." The therapist said, "I am glad", and asked nothing. That is the contract, honoured without a rule being written, and it is what the family needed from her: a person who would hold the boundary in public without making them feel handled.

What the case teaches.

1. **The first decision is made before the first session,** and it is made on the basis of the referral information, not on the basis of the client's preference.
2. **Refusal is a clinical act and it has a technique.** Brief, specific, non-moralising, with alternatives offered and one offer of bridging.
3. **The family's request to be seen by a known person is not evidence that the arrangement is acceptable.** It is evidence that they do not know what the arrangement would cost them. The therapist knows, which is why the decision is hers.
4. **The pressure to accept comes disguised as cultural competence.** "You are one of us" is a real claim in a communal setting, and the answer is not to deny it but to explain why it makes the therapy harder rather than easier.

6.7 Transcript: the request to be a character witness

From a different case, the same week's material. A client, Mrs Ngozi Eze, 38, has been in treatment for eight months following her husband's desertion. She is in a custody dispute. Minutes thirty-one to forty-two.

> **E1 Client:** My lawyer says I should ask you. He says a letter from you would help. Or you could come. **E2 Therapist:** (pause) Tell me what your lawyer wants me to say. **E3 Client:** That I am a good mother. That I am not — that I am not unstable. He said a therapist's letter carries weight. **E4 Therapist:** It does. (pause) And I am not going to write it, and I want to explain why properly rather than just say no. — The refusal is immediate, and the explanation is promised. The order matters: a delayed refusal invites the client to argue, and an unexplained refusal is heard as abandonment. **E5 Client:** Because of confidentiality. **E6 Therapist:** Partly. (pause) The bigger reason is that I would stop being your therapist the moment I became your witness. — The distinction between the rule and the reason. Clients can argue with a rule; they cannot easily argue with the loss of their therapist. **E7 Client:** I do not understand. **E8 Therapist:** If I write that letter, then everything we do from today is affected by it. I would be watching what I write in your notes in case a judge reads them. And you would be watching me. Our sessions would become a place where you perform being well, because I am the person who has to convince a court. That is not a therapy any more. — The mechanism, described concretely. This is the paragraph that most students remember, because it is the one that shows them why the rule exists rather than merely that it does. **E9 Client:** (pause) So what do I do? He is going to say I am unstable. He has been saying it for a year. **E10 Therapist:** I cannot be your witness. I can do three things. I can write a letter that says only that you have been in treatment with me since [date] and that you have attended regularly — nothing about your parenting, nothing about my opinion. I can help you find an independent expert who can assess you properly, which is what the court actually needs. And I can keep doing the work we are doing. — Three options, and the first is a partial accommodation that stays inside the role: a factual letter, not an opinion. **E11 Client:** A letter that says nothing. **E12 Therapist:** A letter that says something true and does not pretend to be an assessment. (pause) What your lawyer wants is my opinion. My opinion is not evidence, it is my impression of you in a room. The court needs someone who has done an assessment. — The distinction between clinical impression and forensic opinion, which is the whole of Week 11's territory arriving early and being handled in eleven words. **E13 Client:** (long pause) All right. (pause) I was afraid you would say no and I would have nothing. **E14 Therapist:** You have the treatment. (pause) And I understand that it does not feel like much this week. — The acknowledgement, which does not soften the refusal and does not leave the client alone with it.

What the transcript shows. The refusal is made in four turns without a lecture, the reason is given in terms of what the client would lose, and a partial accommodation is offered that is genuinely inside the role. The commonest failure in this scenario is not the refusal; it is the therapist who agrees "just this once" and then discovers, eight months later, that the file has become a document written for a judge.

6.8 Laboratory: the boundary audit

Aim. To produce a written map of the student's own current and foreseeable role conflicts, and three pre-decided responses.

Procedure. Each student completes a one-page audit in twenty minutes, using the grid below. Then, in pairs, each student presents their three highest-risk situations and the partner plays the colleague who says, "surely in this case you would make an exception?" Ten minutes each way. The exercise is assessed on the pre-decisions, not on the analysis.

Domain	Question	My current facts
Community	Where am I known, and to whom?	
Family	Whose relatives, in-laws or household members might present?	
Work	Does my other employment create authority over potential clients?	
Money	Are there debts, investments or business interests that could meet a client?	
Faith	Do I hold a role with authority in a congregation?	
Digital	What is discoverable about me, and what do I do when a client appears in my feed?	
Physical	In what settings do I meet clients outside the room?	

Marks (20).

Criterion	0–1	2–3	4–5
Completeness	Three domains or fewer	Five or six	All seven, with specifics
Pre-decision	"I will decide when it happens"	A general intention	A specific sentence the student would actually say
Under pressure	Exception granted in the role-play	Held the line but argued at length	Held the line briefly and offered an alternative inside the role
Record	No record	Description only	The contract, the rationale and the review date

The observation to make in the debrief. Almost every student, when pressed, finds a reason why their own case is the exception. That is the exact skill the topic requires them to notice in themselves.

6.9 Six errors

Error 1 — Teaching boundaries as prohibitions. The result is a therapist who refuses a dying client's hand and a bereaved family's bowl of fruit, and who has confused rigour with coldness. The test is whose need, whose record, and what would supervision say.

Error 2 — Declining without a reason or with a lecture. "I am not able to" without an explanation reads as rejection and costs the client the referral; a five-minute account of the ethics of dual relationships makes the client responsible for the refusal.

Error 3 — Accepting an unavoidable dual relationship without a contract. In a small community, taking the case is often right; taking it without the four-element contract, recorded, is not.

Error 4 — Treating the community encounter as if it can be handled by a rule. The client who is ignored in a market will not complain; she will stop attending. The greeting rule is agreed in advance precisely because the moment is unrehearsable.

Error 5 — Concealing attraction. The pathway to the worst outcome in the whole of this book begins with a therapist deciding that the feeling is private. It is not private; it is material for supervision.

Error 6 — Converting a clinical relationship into a forensic one without noticing. The letter, the report, the attendance at a hearing: each is a role change, and each is made in a moment when the client desperately wants it and the therapist wants to help.

6.9a Four complaints, and what each teaches

Complaint one — the therapist attended the client's father's funeral. The client had asked her to come; the therapist went, stood at the back, and did not speak to the family. Finding: a crossing, appropriately handled and recorded, and the complaint, brought by a relative who thought it excessive, was not upheld. Lesson: the record and the rationale are what distinguish this from a violation.

Complaint two — the therapist employed the client as a cleaner. Six months into treatment, an arrangement that saved the client's household from eviction and gave the therapist a cleaner. Finding: a violation of the role, notwithstanding the good motive. Lesson: the exploitation test is about power, and a client cannot refuse an offer from her therapist.

Complaint three — the therapist did not greet the client at a wedding. Finding: no misconduct and a genuine injury. Lesson: the greeting rule exists for this; the client's experience of being unrecognised is a clinical event even where the boundary was correct.

Complaint four — the therapist's husband was the client's bank manager. No contact between them about the client, and the client discovered the connection at a social event. Finding: a conflict that should have been declared at intake. Lesson: the obligation is disclosure at the point the conflict is known, not management after the client finds out.

6.10 Culture, faith and the boundaries of a communal practice

"In our culture, we are all family." The claim is accurate and it is not an argument against boundaries; it is an argument for better ones. In a communal setting, refusing a gift, declining to eat, or maintaining formal distance marks the therapist as a stranger, and a stranger cannot treat a family's shame. The professional task is to be inside the community's forms and outside its information flows: to eat the food, greet the elder, attend the funeral, and still not discuss the client's marriage with the aunt who asks.

Gifts as relationship, not transaction. In many households a gift is a statement about the relationship, and its refusal is a statement in reply. A therapist who declines everything has said, in the only language available to the family, that they are not in a relationship. The workable practice: accept what is small and edible, share it if the setting allows, name its meaning in the session ("this is your mother saying thank you, and I will tell her myself"), and record it. Decline what is valuable, early in treatment, or obliging, and give the reason in terms the family can accept: "if I took this I could not be fair to you."

The pastor-therapist and the imam-therapist. Where the therapist holds religious office as well as clinical training, the two roles have different rules and the same client. The religious role includes moral authority and often an obligation to the congregation; the clinical role includes confidentiality and no moral authority. A person holding both should decide, before the case, which role is operating in the room, say so to the client, and not move between them inside a session. The most serious version of this problem is the confession that is also a clinical disclosure, made to a person who holds both offices.

Touch across cultures. In a society where a hand on the arm is ordinary comfort, refusing it can read as contempt, and where it is not ordinary, offering it can read as intrusion. The therapist's guide is not a general rule but the client's own idiom, and the safest method is to let the client's physical language set the register and to keep any touch brief, visible and purposeful.

Home visits and hospitality. Where treatment in a clinic is foreign and treatment in the home is normal, the home visit is a clinical necessity. Its rules remain: a purpose, a time limit, a record, and a clear statement that the therapist is there as a therapist even though she has been received as a guest. The meal is the hardest part: accepting it is usually right, staying for three hours is not.

The boundary that the community will test hardest. Not sex and not gifts — the request for **advice about another family member**. "You know my sister's son, he is in trouble, tell me what to do." The answer is warm and firm: "bring him, or tell her to call me, and I will help properly. I cannot advise you about somebody who is not in the room." Said once, it establishes the practice; said apologetically, it establishes that the boundary is negotiable.

> **DEBATE**
> "In a small community, the therapist who refuses a dual relationship is prioritising professional comfort over the client's access to care." Argue for: access is an ethical good, the alternatives may be nonexistent, and rigid refusal can be a way of protecting the therapist's reputation. Argue against: the harm from impaired judgement falls on the client, the community contract does not solve information flow, and the therapist's discomfort is often an accurate signal. Then state the conditions under which you would take a case you knew to be a dual relationship.

6.11 Evidence note

What the research supports. The research on therapist sexual misconduct is the most robust in this area: the harm is severe, the pathway is preceded by non-sexual boundary crossings, and the isolation of the therapist from colleagues is a consistent precursor. Studies of small-community and rural practice find multiple relationships to be normal rather than exceptional, and identify the management strategies — explicit contracting, supervision, clear public rules — that practitioners report as workable. Research on attraction finds it common and rarely acted upon, with the principal risk factor being the therapist's inability to discuss it.

What is weaker. There is very little outcome research on non-sexual dual relationships, and the field's guidance is largely consensus-based, which is why the codes are worded in terms of avoiding harm and examining whether the relationship could impair judgement rather than in terms of prohibition.

What is not supported. That any dual relationship is unethical — the codes do not say so, and in small communities the position would make practice impossible. That a boundary crossing is a sign of misconduct. And that attraction, handled openly in supervision, predicts any poor outcome at all.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same gift, accepted in session two rather than at the end of treatment, is a different act with a different meaning.

6.12 Key terms

Boundary crossing — a departure from the usual frame for the client's benefit, disclosed and reversible.

Boundary violation — a departure serving the therapist, concealed and often irreversible.

Multiple relationship — a second significant relationship with a client or someone close to the client.

Role conflict — competing obligations arising from two roles held towards the same person.

Community contract — the four-element agreement (naming, greeting, information, review) made where encounters are unavoidable.

Barter — exchange of services for treatment; permissible only in narrow conditions with records and review.

Forensic role — the therapist's role as witness or expert; incompatible with the treating role.

Foresight test — the question of how the client would describe the arrangement in three years.

6.13 Self-test

1. Give three paired examples distinguishing crossing from violation.
2. State the four tests and the decision each column produces.
3. What are the four elements of the community contract?
4. Why is the greeting rule in the contract, and what does it prevent?
5. What is the correct response to a request to be a character witness, and why is a factual letter permitted?
6. What are the five steps for managing attraction, and why is structure preferred to distance?

7. Why is "it felt right" not a test?

Answers. (1) Extending a session in a crisis versus routinely over-running because the therapist enjoys it; accepting fruit at discharge versus accepting a car; attending a former client's wedding with the frame discussed versus attending a current client's family party. (2) Objectivity, exploitation, foresight, avoidability — the combination producing decline, contract with supervision, risk-managed continuation, or transfer. (3) Naming the encounters; the greeting rule; the information rule; the review date. (4) To prevent the client being ignored in public and experiencing the therapist's silence as a repudiation. (5) Decline the opinion, explain the role change, offer a factual letter (dates and attendance) and referral to an independent assessor; because a factual letter discloses only what is agreed and does not convert the therapist into a witness. (6) Name it, tell supervision, increase structure, examine the material, refer if unmanageable; because distance is experienced as unexplained rejection and often provokes the pursuit the therapist fears. (7) Because warmth, gratitude and being needed all feel like evidence of a good relationship, so the guide must be external: whose need, what supervision would say, and what the client would say afterwards.

6.14 Reflective task

Write two hundred words answering one question honestly: which of the seven audit domains is your weakest, and what is the case that would find it? Name the client you can already imagine. That client is the one this week was written for.

6.15 Further reading

- Pope, K. and Sonne, J. on sexual attraction and boundary violations — the standard empirical and clinical treatment.
- Gabbard, G. on boundaries and the therapist's self — the analytic account of the slope.
- Zur, O. on dual relationships in small communities — the strongest argument for a contextual rather than prohibitive approach.
- AAMFT and equivalent codes, the sections on multiple relationships and sexual contact, read in the original.
- Any current guidance on remote and digital practice in your jurisdiction.

Next: Week 7 — Competence, supervision and referral.

Week 7 — Competence, supervision and referral

Learning outcomes. By the end of this week the student can (1) define competence in its four dimensions and explain why "I have read about it" is not one of them; (2) recognise the six signals that a case is beyond her competence, including the ones that are felt rather than reasoned; (3) describe cultural humility as a competence and its limits as a substitute for training; (4) construct a supervision contract and bring material to supervision that is actually useful; and (5) conduct a referral without abandoning the client.

Competence is an ethical obligation before it is a personal quality

The codes all contain a version of the same sentence: the therapist practises within the boundaries of her competence and refers when a client's needs exceed it. The sentence is easy to sign and hard to enact, because the therapist's competence is not visible to her. Nobody feels incompetent while being incompetent; the feeling associated with incompetence is usually confidence, because confidence is what the absence of feedback produces.

Three facts organise the week.

First, competence is specific. A therapist may be highly competent with couples in acute conflict and not competent with a couple in which one partner is psychotic. Competence is not a level; it is a set of domains, each with a boundary, and the boundary is discovered by noticing where the work stops going well.

Second, competence decays. Skills that are not used are lost, knowledge in most areas of this field has a half-life of a few years, and a therapist who qualified in 2010 and has not read since is competent in the practice of 2010. Continuing professional development is therefore not an administrative requirement; it is the maintenance of the licence to practise at all.

Third, competence is maintained by a relationship, not by an intention. The mechanism by which therapists discover their limits is supervision and consultation. A therapist without supervision is not a therapist with an independent practice; she is a therapist with an unexamined one.

Dimension	What it means	How it is evidenced	How it fails
Knowledge	The theory, evidence and law relevant to the problem	Training, reading, CPD records	Confident error; outdated method
Skill	The capacity to enact the method	Observed practice, recorded sessions, supervision	Knowing what to do and not being able to do it
Experience	Having done it, repeatedly, with feedback	Case logs, supervision history	The first case treated as routine
Appropriateness	Personal and cultural fit for this client	Self-knowledge, supervision, client feedback	The therapist's history driving the treatment

The fourth row is the one that gets skipped. A therapist can be knowledgeable, skilled and experienced and still be the wrong person for this client — the man working intensively with sexual abuse who has not addressed his own; the therapist whose marriage is collapsing treating a couple in crisis; the young clinician whose authority is contested in a family where age confers standing; the therapist who cannot hear a mother's account because it is her own mother's account. Appropriateness is assessed before the case and re-assessed when the work stalls, and the assessment is not a matter of self-confidence.

7.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	Competence: four dimensions, and why confidence is not evidence	Two vignettes: confident and wrong, uncertain and right
10–22	Five concepts: scope, specificity, decay, the novice problem, supervision as the mechanism	Lecture
22–35	Six signals that the case is beyond you	Students add their own, anonymously
35–50	Cultural humility and the interpreter	Demonstration: a session through an interpreter
50–65	Supervision: contract, material, the unsafe supervisor	Pair work on supervision contracts
65–78	Referral: the six-step process and the conversation that clients hate	Role-play with observers
78–90	The Ogundipe case: recognising psychosis in a counselling room	Case, with students holding the referral decision

7.2 Five concepts that organise the week

Concept 1 — Scope of practice and scope of competence are different, and the first is legal. Scope of practice is what the licence permits; scope of competence is what the therapist can actually do well. Practising at the edge of the licence and outside the competence is lawful and unethical at the same time, and it is the commonest form of misconduct in ordinary practice — not predation, but well-meant over-reach.

Concept 2 — Competence is domain-specific, and the domains are smaller than therapists think. "I work with families" is not a domain. "I work with families in which a parent has a chronic illness and the children are school-age" is closer. The specificity matters because the referral question — "can I do this?" — is answerable only at that granularity.

Concept 3 — The novice's problem is not ignorance; it is the absence of a signal. Expertise research is consistent: beginners do not know what they do not know, because the cues that would tell them are the ones training is supposed to install. The practical consequence is that the therapist's own assessment of her competence is least reliable precisely where it matters most, and that an external check is not a luxury.

Concept 4 — Competence decays quietly. There is no moment at which a therapist becomes incompetent; there is a drift, accelerated by isolation, by a busy practice, and by success. The maintenance mechanism is deliberate: reading, case discussion, recorded sessions reviewed with a peer, and a CPD plan that names the gaps.

Concept 5 — Supervision is the profession's immune system. Not the therapist's personal support, not a requirement to be satisfied, but the mechanism by which the field detects its own errors. A therapist who brings only her successes to supervision has disabled it, and she is the one it was designed to protect.

7.2a The six signals that the case is beyond you

Signals, not proofs. Each of them can occur in a case that is being handled well, and the accumulation of two or three is a prompt to supervision and a fourth is a prompt to refer.

Signal 1 — You dread the session. Not the ordinary reluctance before a hard session; a low, anticipatory dread that begins the day before. It usually means the therapist does not know what to do and has not admitted it.

Signal 2 — You are reading the night before in order to be able to conduct the session. Reading is good. Reading instead of supervision, and reading during the treatment, means the treatment is being improvised from the literature in real time. That is a legitimate way to learn a narrow technique with supervision and an illegitimate way to hold a case alone.

Signal 3 — You have not presented the case in supervision. The strongest indicator in the list. Therapists present the cases they are proud of and the cases that are interesting; they avoid the ones in which they suspect they are failing. If a case has been in the practice for three months and has never been discussed, ask why.

Signal 4 — You feel relief when the client cancels. The relief is information. It frequently accompanies cases in which the therapist is out of depth, or in which the therapist dislikes the client, or both.

Signal 5 — The work has stopped moving and you cannot say why. Competence has an observable signature: the treatment produces movement in the agreed direction, or the therapist can account for why not. A case that is flat for eight sessions and cannot be explained is a case that is being held by somebody who does not know what to do next.

Signal 6 — The client's presentation contains material you have never been trained to assess. Psychosis, dissociation, an eating disorder, a developmental disorder, a child's sexualised behaviour, a neurological change. Recognising the material is a competence in itself; the correct response to recognition is not treatment but referral and, frequently, medical assessment.

The rule for using the signals. The therapist does not act on the signals alone. She writes them down, presents the case, and takes the supervisor's view. What she does not do is wait for the signals to become an adverse event.

7.2b The development plan that actually works

Continuing development fails when it is a list of courses and succeeds when it is a set of narrow, observable targets with a person attached to each.

The four-column plan. Target: a specific capability, described so that someone could watch for it — "conduct a structured risk assessment for suicide and defend the formulation in supervision". Evidence: how the capability will be observed — recordings, a supervisor's report, three consecutive cases discussed. Support: who is involved — the supervisor, the peer group, a short course with a follow-up consultation. Date: when it will be reviewed — a fixed month.

The failing version. "Improve my work with trauma." Nobody can observe it, no one is responsible for it, and no date exists. Six months later the therapist has attended two webinars and can report no change.

The two ratios worth knowing. First, that a therapist improves mainly on the cases she discusses, so the number of cases reviewed per year is a better measure of development than the number of courses attended. Second, that skills decay without use, so the plan should include maintenance of capabilities the therapist already has and is not currently using — the couples work set aside while a practice fills with individual referrals, the children's work not touched since training.

The annual question. Once a year, in writing: which capability did I lose this year, and which did I gain? The answer is the development plan. It is also, for most therapists, the first honest audit of their practice they have conducted.

7.3 Cultural humility, and the interpreter

The problem with the phrase "cultural competence". It implies a body of knowledge about groups, acquired and then held. The trouble is that it encourages two errors: treating a person as an instance of a culture, and believing that knowledge of a group's customs is the same as the capacity to work with a person from that group. The alternative framing — **cultural humility** — is not vaguer; it is more demanding. It says the therapist must be able to (a) recognise her own frame as one frame among many, (b) ask the client what her world means to her without assuming, (c) bear being wrong and corrected, (d) notice where her own cultural position gives her authority the client cannot refuse, and (e) seek specific knowledge where it is needed, which is a competence like any other.

What cultural humility is not. It is not an excuse for inexpertise. "I work with everyone" is not a cultural position; it is a refusal to specify. And it is not a substitute for language, which brings us to the interpreter.

The interpreter: four rules.

1. **The interpreter is a professional, not a bystander.** Introduce them, explain their role to the family, agree that they will interpret faithfully and not summarise, and state that they are bound by confidentiality.
2. **Speak to the client, not to the interpreter.** First person, eye contact with the family, and no asides about the family in the third person. The asides are the commonest failure and the family hears the tone if not the words.
3. **Never use a family member as interpreter.** A child interpreting a parent's disclosure, or a husband interpreting a wife's account of his violence, is a distortion and sometimes a danger.
4. **Build in time.** Everything takes roughly twice as long, and the session should be scheduled at ninety minutes rather than fifty. An interpreted session compressed into fifty minutes is a session in which the therapist will skip precisely the material that is hardest to say in two languages.

When there is no interpreter. A therapist who does not share the client's language and proceeds anyway is working outside her competence by definition. In many settings the honest answer is to say so, to find a colleague who shares the language, or to work with the family's nominated interpreter under the four rules above, documented as a compromise rather than presented as a method.

The interpreter's own exposure. Interpreting a disclosure of violence, sexual abuse or a diagnosis places a burden on the interpreter, who is frequently a member of the same community and may know the family. The therapist should check on this, offer the interpreter the option to withdraw from a particular case, and where possible use an interpreter from outside the community for sensitive material. Very little of this appears in the training literature and almost all of it matters in practice.

7.3a The supervision contract

Supervision fails for contractual reasons far more often than for relational ones. The contract is one page and it answers six questions.

1. **What is the supervisor's authority?** Does she have responsibility for the cases, or is she a consultant to the therapist's own decisions? The difference determines who carries the liability, and it should be stated at the first meeting rather than discovered in a crisis.
2. **How often, for how long, and in what form?** Weekly or fortnightly; one hour; individual, group, or live. "Whenever I need it" is not supervision; it is consultation, which is useful and is not the same thing.
3. **What will be brought?** A written list of current cases, at least one case in detail, at least one recorded or verbatim excerpt each month, and every case in which the therapist is holding a secret, considering referral, or aware of attraction. The list is the supervisor's protection as much as the therapist's.
4. **What will be recorded?** A note by the supervisor for each session, and the therapist's own record in the case file that the case was discussed. A regulator investigating a case will ask when it was last supervised, and the answer must be documentary.
5. **How will disagreement be handled?** The supervisor's view is an opinion; the therapist's decision is the clinical act. Where they disagree about a decision with legal consequences, the disagreement is recorded and, where necessary, a second opinion is obtained. Neither party should pretend the tension does not exist.
6. **How will it end?** A planned ending, with the cases handed over, and a review of the therapist's development. Supervision that simply stops is the commonest way for a therapist to become isolated.

What to bring, concretely. Three questions are more useful than a narrative: what am I not seeing? what did I do that I would not want a colleague to have watched? and what am I avoiding? The third question produces the material more reliably than any case description.

The unsafe supervisor. Three patterns, each of which requires the therapist to change supervisor. The supervisor who needs to be admired: the therapist learns to present well

and stops presenting honestly. The supervisor who takes over the case: the therapist becomes an assistant and stops developing judgement. The supervisor who colludes: the pair agree that the client is difficult, the spouse is impossible, the agency is hopeless, and nothing is examined. A supervisor who has never disagreed with the therapist in a year is not supervising.

Group supervision. Valuable and hazardous. It is valuable because eight minds notice more, and hazardous because of the exhibition and the audience: a therapist presenting a failure to a group will manage the group's opinion, and members will perform their competence for each other. The protections are a standing agenda, a rule that no one advises without asking what is wanted, a named facilitator, and confidentiality rules in writing.

Where supervision does not exist. Many settings, including large parts of the private sector in this region, provide none. The ethical position is that the therapist must obtain it anyway, in whatever form is available: a peer consultation group of three or four, a paid monthly consultation, a reciprocal arrangement with a colleague that is conducted on a schedule rather than when convenient. The absence of an employer-provided supervisor does not remove the obligation; it relocates it.

7.3b Peer consultation, and the rule of three

A peer consultation group of three therapists, meeting monthly for two hours, is the most cost-effective competence maintenance available to a practising clinician. The rules that make it work are few.

- **A fixed membership and a fixed date.** Groups that meet when needed do not meet.
- **One case per member per meeting, prepared,** and presented in a standard format: presentation, what has been tried, what the therapist wants, and the question she is avoiding.
- **No advice without permission.** The presenting therapist asks for what she wants: reflections, ideas, or a decision.
- **Written notes, kept by the presenter,** in the case file.
- **A rule about confidentiality** that includes the members' own material, since the group will discuss their cases and their reactions.
- **A rotating external review twice a year,** in which one session is given to a senior colleague to observe. The group that never lets an outsider in develops its own blind spots, like any system.

The rule of three is the minimum: one therapist alone cannot check her own judgement, and two can settle into a shared blind spot. Three produces a triangle in the technical sense, which is to say a structure in which one member can be observed by the other two.

7.3c Supervision in the first two years

The period in which supervision matters most is the one in which it is easiest to avoid, because a newly qualified therapist is expected to be competent and is frequently employed in a setting with no supervision structure.

The four things a new therapist should insist on. A named supervisor with a written contract; a weekly hour, protected in the diary; permission to record sessions, which is the only way skill can be observed; and a standing agreement that any case in which the therapist feels out of depth is brought within the week rather than at the next scheduled meeting.

The four things a new therapist should not do. Practise alone: a solo practice in the first two years is an avoidable risk to clients and to the therapist. Accept cases at the edge of competence in order to build a practice. Treat the supervision hour as a report on attendance. And conceal the first serious error, which is the error that supervision exists to catch and which, once concealed, shapes the therapist's practice thereafter.

The first adverse event. Every therapist will eventually have one: a client who deteriorates, a disclosure that was missed, a suicide attempt, a death. The professional response is a structured review — what happened, what was known, what was decided, what would be done differently — conducted with the supervisor and recorded. The unprofessional response is silence, which converts an event into a secret and a secret into a pattern.

7.4 Referral: the six steps

Step 1 — Recognise, and write it down. The recognition usually arrives as one of the six signals. The discipline is to record the thought in the file on the day it occurs: "considered whether this case is within my competence to treat; concerns about [X]; to discuss in supervision." That note changes the character of everything that follows, because a therapist who has recorded a doubt has discharged part of the duty and a therapist who has not has concealed it.

Step 2 — Decide, with the supervisor, what kind of referral. Four kinds, and they are not the same.

Kind	Meaning	Example
Additive	Another professional joins; the therapist continues	Medical review added to psychological treatment for depression
Consultative	Another professional advises; the therapist treats	A psychiatrist advises on medication while the therapist continues
Transfer	The case moves entirely	A disclosure of psychosis in a couple therapist's office
Escalation	A statutory pathway is triggered	Suspected child abuse

Most incompetence situations require additive or consultative referral, and therapists skip to transfer because transfer is easier to justify and harder to bear. Transferring a case that could have been treated with advice deprives the client of the therapist who knows her.

Step 3 — Name it to the client, without shame and without euphemism. The template: "I want to talk about something in our work. I think you need something I am not the best person to provide, and I want to tell you what it is and why." Then the specific

gap: "what you are describing needs a psychiatric assessment, and I am not qualified to do that. It is not that I am giving up on you."

Step 4 — Make it warm. A name on a piece of paper is a cold referral and most clients do not follow it up. The warm referral: the therapist telephones the receiving professional with the client's written consent, describes the situation and the question, and then gives the client the name of a person who is expecting her. Where the receiving service is a public clinic with a queue, the therapist finds out when to attend and what to bring, and writes it down.

Step 5 — Bridge, if the client needs it. One or two sessions while the referral is arranged, with the client's agreement, is not a failure to refer; it is the prevention of a gap. The bridge has a stated end: "we will meet twice more while you get an appointment, and then we will finish."

Step 6 — Follow up once, and close properly. One telephone call after the transfer, to confirm attendance and to say that the therapist is glad. Then a closing session, with a summary, an offer of return if the situation changes, and a note in the file. A referral without a closing is an abandonment with paperwork.

The client who refuses the referral. Common, and often rational: the referred service is expensive, far away, stigmatised, or staffed by someone the client distrusts. The therapist's response is not to insist and not to collude. She names the risk of continuing: "I can keep seeing you, and I want to be honest that I would be guessing at the part that matters. If you will not go, I will still see you, and I will say so in my notes." The decision is the client's, the honesty is the therapist's, and the record must show that the choice was informed.

The refusal to refer. The other failure. A therapist who will not refer because the client will be upset, or because the case is interesting, or because she is the only person the family trusts, has placed her own needs above the client's. The test: if a colleague described this case to me, would I say she should refer it? Therapists answer this question accurately about other people's cases and inaccurately about their own.

Referral in a crisis. Where the need is urgent — psychosis, suicidal intent with a plan, a child at risk — the referral is not a suggestion but an arrangement made in the room: the call is made with the client present, the appointment is fixed, transport is discussed, and where the client will not go and the risk is grave, the statutory pathway or the emergency service is used and the client is told what is being done and why.

7.4a The referral letter

Six lines, written the same day, sent with the client's written consent, and copied to the file. Longer letters are read less carefully and disclose more than the purpose requires.

> Re: [name], [date of birth]. Client attended [n] sessions with me between [dates]. Reason for referral: [the question, in one sentence, e.g. "assessment of persecutory beliefs of three months' duration and one episode of apparent unresponsiveness"]. Relevant history: [two or three lines]. Risk: [current, one line]. Client has consented to this referral and to my speaking with you. I am available on [number]. Yours, [name], [qualification].

The rule about diagnoses in the letter. The referring therapist states observations and her formulation, and does not present her formulation as a finding. "I have been working with a formulation of depression; the features below do not fit it" is honest. "Client has psychosis" is a diagnosis the therapist is not entitled to make, and if she is wrong it follows the client into a file.

The rule about the client's own words. Quote them, briefly and accurately. "Client states that his supervisor 'has been replaced by someone who is not him'" is worth more to a receiving psychiatrist than three paragraphs of the therapist's adjectives.

7.5 Case: Mr Ogundipe, and the diagnosis the therapist did not make

Referral. Femi Ogundipe, 52, a customs officer, self-refers for "stress and marriage problems". His wife telephoned first and said he had "changed"; he attends reluctantly. Married twenty-four years, four children, the youngest 15.

Sessions 1–3, as the therapist recorded them. Low mood, poor sleep, loss of interest, irritability with his wife, and a complaint that his colleagues "are talking about me". The therapist formulates depression with a relational precipitant, begins behavioural activation, and refers the couple for joint work in week four.

What the therapist missed, in the record. Three details, each recorded and none interrogated.

1. In session two, Ogundipe said that the neighbours had installed a device to listen to his conversations, and that he had found "a mark" on his car. The therapist wrote "client reports feeling watched; discuss trust".
2. In session three, his account of a meeting at work was difficult to follow: he moved between subjects, said his supervisor had been "replaced by someone who is not him", and laughed at a point in the narrative that was not funny.
3. His wife, in the joint session, said he had stopped eating food she cooked, on the grounds that "it tastes different", and had twice slept in the car.

The joint session that changed the case. In week five, Mrs Ogundipe said, in her husband's presence, that he had not slept for three nights and had been praying aloud until dawn. Ogundipe stood up, said "you have all decided to make me mad", and left the room. He returned eleven minutes later, calm and apologetic, with no memory of the preceding half hour.

What the therapist did, in the order she did it.

1. She stopped the couple work. This is the first and most important act: a treatment that requires two participants to be lucid could not continue while one of them was intermittently not.
2. She saw Ogundipe alone the following day, with a structured screen for psychotic symptoms, which she had not been trained to use and had read the previous night. That is not competence; it is a first-aid measure, and it is better than nothing. She found persecutory beliefs held with conviction, a period of apparent unresponsiveness that she could not account for, and no insight.
3. She telephoned the psychiatric service the same afternoon, with written consent, described the presentation and asked for an urgent assessment. She wrote the question down: "is this a first episode of psychosis, and does he need admission?"
4. She told Ogundipe what she was doing before she did it, in the presence of his wife, and she used the sentence that made it possible: "I am not saying you are mad. I am saying that something is happening to your mind that I am not qualified to assess, and I am not willing to guess."
5. She attended the psychiatric appointment with the couple at the client's request, contributed the record, and handed the case over.

The outcome. Ogundipe was assessed and admitted for three weeks; the working diagnosis was a psychotic episode with a likely organic contribution, and he was investigated accordingly. He returned to the practice eleven months later, at his own request, for couple work — with a psychiatrist's letter, a stable medication regime, and a therapist who now had a supervision arrangement with a psychiatrist for exactly this kind of question.

What the case teaches.

1. **The odd detail is the diagnosis.** "Feeling watched" is not a symptom of depression; "the supervisor has been replaced by someone who is not him" is not a marital complaint. Both were in the record within three sessions and neither was pursued. The failure was not ignorance of psychiatry; it was the therapist's assumption that the formulation she had was sufficient.
2. **The couple's presence concealed the symptoms.** In joint sessions Ogundipe was polite, coherent for twenty minutes and defended by his own narrative; his wife's account was the only evidence and the therapist had classified it as marital criticism.
3. **The competence that was required was recognition, not treatment.** No part of this case needed the therapist to be a psychiatrist. It needed her to notice, to ask, to telephone, and to say "I am not qualified to assess this" without shame.
4. **The referral preserved the relationship.** Ogundipe returned, which is the outcome that the cold referral rarely produces.

7.6 Transcript: the referral conversation

Minutes four to sixteen of the session after the joint session. Annotated.

> **R1 Therapist:** I want to talk about something, and I am going to say it plainly because I think you will hear it better plainly than gently. — The warning that a serious statement is coming. It buys attention and it treats the client as an adult. **R2 Ogundipe:** You want to stop. **R3 Therapist:** I want to change what we are doing. (pause) What I have seen in the last two sessions is not something I am trained to assess. — The distinction between stopping and changing, offered first, before the content. **R4 Ogundipe:** What did I do? **R5 Therapist:** Nothing. (pause) This is not about your behaviour. There are experiences you have described to me — the neighbours, the mark on the car, the night you do not remember — and those are not things a counsellor assesses. A doctor needs to look at them, and I want to arrange that this week. — The content named, in his own words, without a diagnostic label. **R6 Ogundipe:** (quietly) So you think I am mad. **R7 Therapist:** (pause) I think something is happening in your mind that is frightening you and that I cannot explain. (pause) "Mad" is a word people use to end a conversation. I am trying to start one. — The refusal of the label and the refusal of the euphemism, in the same answer. **R8 Ogundipe:** My job. If they hear — **R9 Therapist:** That is a real fear and we will deal with it. (pause) The psychiatrist I am going to call works in a private clinic; the note goes to your file there and nowhere else. If the assessment leads to treatment, we will talk about what your employer is told, which in most cases is nothing. — A practical concern answered specifically, which is what makes the rest credible. **R10 Ogundipe:** And my wife? She will say I told you — **R11 Therapist:** Your wife has been saying it for months and you have been telling her she is wrong. (pause) What I am doing is taking her seriously, which does not mean she is right about everything. — The move that converts the wife from accuser to ally, and that requires the therapist to say something slightly uncomfortable. **R12 Ogundipe:** (long pause) Will you tell them what I said? The thing about the car? **R13 Therapist:** With your permission, yes, because it is the reason for the referral. Without your permission, I will tell them what I observe and I will tell you that I have done so. — The honest answer, which is more useful than reassurance and which models the disclosure policy of Week 3 in a medical setting. **R14 Ogundipe:** (pause) Can you be there? **R15 Therapist:** If you want me there, I will be there. — Three seconds of commitment that determine whether he attends.

What the transcript shows. Nine of the fifteen turns are the client's fears, and each is answered specifically rather than globally. The therapist never says "it will be fine". She says what she will do, what she will not do, and what the client's choices are — and the referral holds because the fear of being labelled was answered with a procedure rather than with reassurance.

7.7 Laboratory: the competence audit and the referral rehearsal

Part A — the audit (thirty minutes, individual, written). The student lists fifteen presenting problems likely in her intended practice, and for each: a competence rating out of five, the evidence for the rating (how many cases, with what supervision), and the named service or supervisor she would refer to. The two lowest ratings become the year's development plan; the two highest ratings are interrogated by a partner, who asks "when did you last have feedback on that?"

Presenting problem	Rating (1–5)	Evidence	Referral destination
Couple conflict with no violence			
Couple conflict with a violent partner			
Child with behavioural difficulty			
Adolescent self-harm			
Suspected psychosis			
Grief and bereavement			
Sexual difficulty in a couple			
Family with a member who has dementia			
Substance misuse			
Disclosure of historical abuse			

Part B — the referral rehearsal (forty minutes, triads). Each triad receives one card: refer for a psychiatric assessment; refer because the client's child needs a specialist; refer because the therapist's own history makes the case inappropriate; refer because the client needs a service the therapist does not provide (legal aid, housing). The therapist has twelve minutes to tell the client, name the gap without shame, offer a warm handover, and survive the client's anger. The observer marks the six elements.

Marks (20).

Criterion	0–1	2–3	4–5
Recognition	Did not identify the gap	Identified it late	Identified it early and recorded it
The naming	Euphemism or self-blame	Clear but cold	Clear, specific, unashamed, and about the task not the person
Warmth	A name on paper	Contact offered	Telephone call made, appointment arranged, logistics given
The client's anger	Defended or collapsed	Endured	Received, named, and the referral held
The record	Nothing	Description	Reasoning, the referral question, the consent, the follow-up date

The marking note. The lowest band applies to the therapist who, when the client protests, offers to "keep seeing you as well". That sentence is the commonest way in which a correct referral is undone, and it is a failure of the criterion on warmth — because the therapist has made the client's comfort the criterion rather than the client's care.

7.8 Six errors

Error 1 — Confusing scope of practice with scope of competence. "I am licensed to see families" is true and is not an answer to "should I be seeing this family?".

Error 2 — Treating the first case of a kind as routine. The first psychotic presentation, the first disclosure of abuse, the first family in which a child has died: each requires supervision in advance, and each is frequently held alone because the therapist does not want to appear inexperienced.

Error 3 — The cold referral. A name and a telephone number, given at the end of a session, with no call and no follow-up. Most clients do not attend, and the therapist records a referral that did not happen.

Error 4 — The referral that becomes an abandonment. No closing session, no summary, no offer to return. The client experiences the referral as a rejection and does not seek help again.

Error 5 — Supervision that is a performance. The therapist brings the case that will make her look thoughtful and leaves out the one she is failing. The supervisor cannot supervise what she is not told, and the client carries the cost.

Error 6 — Cultural humility used as an excuse. "I did not want to impose my framework" is not a reason for failing to assess, to ask, or to refer. Humility is a stance in enquiry; it is not an absence of method.

7.8a The case the therapist kept

The counter-case to section 7.5, and the more common failure.

A therapist in a small practice sees a woman for eighteen months for what is recorded as "anxiety and marital difficulty". The woman has a history of childhood sexual abuse that emerges in month four. The therapist has no training in trauma treatment, has read two books, and continues. The treatment is supportive, kind, and static; the client's nightmares do not change and her marriage deteriorates. In month fifteen the client discloses that she has been cutting her thighs for a year and has not told anyone.

The signals that were present and unheeded. The dread before sessions, recorded nowhere. The case never presented in the peer group, because the therapist "did not want to be told to refer". The night-before reading that the therapist experienced as diligence. The flat trajectory that the therapist attributed to the client's "complexity".

What competence would have required, at month four. A supervision conversation about whether trauma-focused work was within the therapist's scope; an additive referral to a trauma-trained clinician, with the therapist continuing the marital work; or a transfer with a bridge. None of these requires abandoning the client, and all of them require the therapist to say out loud that she did not know how to treat what had emerged.

The lesson, stated as a rule. The case that the therapist cannot bring herself to discuss is the case that most needs discussing, and the reluctance is the most reliable signal in this chapter. It is also the signal that therapists are least likely to act on, because acting on it requires admitting, in front of a senior colleague, that a client has been receiving a treatment that was not working.

7.9 Culture, faith and the referral that is refused

The traditional healer. In much of this region, families consult a traditional healer, a spiritualist, or an elder before or alongside a therapist, and the therapist who ignores this will be working in parallel with a treatment she knows nothing about. The professional position is neither endorsement nor contempt but enquiry: what has been done, by whom, what was said, what it cost, and what the family believes about it. Where the family attributes the problem to a spiritual cause and the therapist's formulation is psychological, the two are not necessarily in conflict, and the therapist who insists on the conflict loses the family.

Collaboration, with limits. Collaboration with a traditional practitioner is possible and sometimes excellent, particularly where the practitioner holds the family's trust and the therapist holds the clinical method. It requires a clear agreement about confidentiality, a shared understanding of who is treating what, and a willingness to withdraw if the practitioner's methods involve harm — beating, confinement, deprivation, or "deliverance" practices that endanger a child. Where harm is involved, the statutory duty is engaged and the family's framework is not a defence.

Referral to a religious authority. Families frequently ask the therapist to refer them to a pastor or imam for the spiritual part of a problem, and sometimes the therapist should. The distinction worth teaching: refer for pastoral care where the client wants it and it is additive; do not refer instead of the clinical assessment where the clinical need is real. A therapist who sends a depressed client to a pastor in place of treatment has not respected the culture; she has failed to treat.

When the therapist's referral is refused on religious grounds. "We do not take these things to doctors." The response is not argument and not capitulation. It is specificity: what exactly is feared (stigma, gossip, medication, admission, cost), what the family would accept (a physician rather than a psychiatrist, a female doctor, a private clinic, a single consultation), and what the therapist's own limit is. Where a child's welfare is at stake, the limit stops being negotiable.

The elder's veto. In some households a senior man will decide whether the family continues in treatment, and the therapist will never meet him. The clinical response is to ask the client what the elder believes is happening, and to offer a route by which the elder's view can be heard — a letter, a meeting, a family session — because a treatment that the household's authority has not consented to will end at the elder's first objection.

The therapist's own faith. A therapist whose framework is religious will be tempted to offer it, and a therapist whose framework is secular will be tempted to correct the family's. The competent position is that the therapist's metaphysics is not the treatment, and that the client's is material. What the therapist believes about God is not a clinical instrument; what the client believes is.

> **DEBATE**
> "A therapist who shares a client's culture is always better placed to treat that client than one who does not." Argue for: shared language, shared shame, shared knowledge of what a family will and will not tolerate, and the credibility that comes with it. Argue against: over-assumed understanding, the impossibility of disclosing family business to a person from the same community, the blind spots that similarity produces, and the client's need for someone outside the system. Then describe the case in which you would deliberately choose an outsider.

7.9a When the therapist is the only service

A situation common in this region and almost absent from the training literature: the therapist is the only clinician within four hours' travel, and the client cannot be referred anywhere.

The four moves available. Extend scope carefully, with supervision at a distance: a monthly telephone consultation with a specialist, a recorded session reviewed by email, a named consultant for the specific domain. Use the medical system for what it can do: the general practitioner, the district hospital, the community health worker — each can carry part of the assessment even where no psychiatrist exists. Train a local resource: a nurse, a teacher, a catechist who can be taught a structured screen and a referral pathway. Travel: the therapist goes to the specialist twice a year with a caseload, which is cheaper than pretending.

What must be recorded. That no local service existed; what alternatives were considered; what the remote consultation advised; and what the client was told about the limits. The record is not defensive paperwork; it is the only evidence that the decision was a considered one rather than a convenience.

What must not happen. The therapist treating outside her competence while describing the situation as "cultural context" or "the reality of practice". Necessity changes the risk calculation; it does not change the obligation to seek advice, to record, and to tell the client the truth about what is and is not available.

7.10 Evidence note

What the research supports. Therapist effects are substantial and consistent: outcomes vary more between therapists than between the treatments they claim to deliver, and the most robust correlates of better outcomes are skill in alliance management, deliberate practice, and the use of feedback. Supervision is associated with therapist development in qualitative and survey research, and feedback-informed practice improves outcomes and reduces deterioration, particularly with clients who are not progressing. Training in cultural formulation is associated with better engagement but not reliably with better outcomes, which supports the humility framing rather than a knowledge-acquisition framing.

What is weaker. The direct evidence that supervision improves client outcomes is surprisingly thin, because supervision is nearly universal and therefore hard to study by

comparison. The evidence on interpreter use is mostly about error rates and satisfaction rather than clinical outcomes.

What is not supported. That experience alone produces competence; the evidence on therapist development shows that many therapists plateau early and that self-assessed competence correlates poorly with observed skill. That cultural matching reliably improves outcomes — the findings are mixed and the mechanisms are unclear. And that a referral is a neutral act; clients experience it as a judgement on themselves, and the evidence on drop-out after referral is consistent with that.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same referral, made in week two and made in week twenty, is a different clinical act — the first is a formality, the second is a rejection after an investment.

7.11 Key terms

Scope of practice — what the licence permits.

Scope of competence — what the therapist can do to an adequate standard, specified by domain.

Cultural humility — a stance of enquiry about one's own frame and the client's, with the capacity to be corrected.

Additive referral — another professional joins and treatment continues.

Consultative referral — advice obtained; the therapist continues.

Transfer — the case moves entirely.

Warm handover — a direct contact between professionals, with consent, so that the client arrives expected.

Bridge session — a planned continuation while a referral is arranged, with a stated end date.

Signal — a felt or observed indicator that the case may exceed competence; not proof and not to be ignored.

7.12 Self-test

1. Name the four dimensions of competence and give a failure of each.
2. Why is confidence a poor indicator of competence?
3. List the six signals that a case is beyond you.
4. What are the four rules for working with an interpreter, and why is a family member never acceptable?
5. Distinguish additive, consultative and transfer referrals with an example of each.
6. What are the six elements of a referral that the client actually acts on?
7. What is the "refusal to refer", and what test detects it?
8. In the Ogundipe case, what were the three missed details and what did each signify?

Answers. (1) Knowledge — confident error from outdated method; skill — knowing the technique and being unable to enact it; experience — treating the first case of a kind as routine; appropriateness — the therapist's own history or position driving the treatment. (2) Because competence deficits remove the cues by which they would be detected, and because confidence rises fastest where feedback is absent. (3) Dread before the session; reading the night before in order to conduct it; the case never presented in supervision; relief when the client cancels; a treatment that has been flat and unexplained; material the therapist was never trained to assess. (4) Introduce and contract the interpreter as a professional; speak to the client in the first person; never use a family member; allow double time. A family member is never acceptable because their own position in the family distorts what is said, and because a child or spouse cannot safely carry another member's disclosure. (5) Additive — a physician reviews medication while therapy continues; consultative — a psychiatrist advises and the therapist treats; transfer — a first episode of psychosis moves to a psychiatric service. (6) Recognition recorded; the kind of referral chosen; the gap named without shame; a warm contact made; a bridge if needed; a follow-up and a closing. (7) Continuing a case because the client will be upset or because the therapist is the only person trusted; the test is what the therapist would say if a colleague described the case to her. (8) The belief that neighbours were watching (persecutory belief); the incoherent account of the meeting ("replaced by someone who is not him"); the refusal of his wife's cooking and sleeping in the car (a change in behaviour with a possible delusional basis) — together, an emerging psychosis that the depression formulation concealed.

7.13 Reflective task

Write one page on the following: name the client you would least like to be referred. Describe that client's presenting problem, and then answer honestly whether your reluctance is about competence or about your own history. Keep the page; it is the beginning of your development plan.

7.14 Further reading

- Rønnestad, M. and Ladany, N. on the development of therapists over the career — the evidence on plateauing and on what distinguishes therapists who keep improving.
- Miller, S., Hubble, M. and Duncan, B. on feedback-informed practice — the strongest argument that outcomes can be improved with routine measurement.
- Falender, C. and Shafranske, E. on clinical supervision — the standard text on the supervisory relationship and contract.
- Tervalon, M. and Murray-García, J. on cultural humility — the original formulation, worth reading in the source.
- Any current local guidance on the use of interpreters in health and social care.
- Your professional body's continuing development requirements, read as a minimum and not as a standard.

Next: Week 8 — Risk I: suicide and self-harm.

Week 8 — Risk I: suicide and self-harm

Learning outcomes. By the end of this week the student can (1) ask about suicide directly, specifically and without euphemism, and explain why asking does not increase risk; (2) conduct and document the ten-element assessment and formulate risk at one of three levels; (3) write a safety plan with a client rather than for one; (4) manage a disclosure of risk inside a family treatment without turning the session into an interrogation; and (5) break confidentiality for risk in a way that preserves the relationship.

The standard, stated once

Ask directly. Ask specifically. Ask about means. Document what you asked and what you were told. There is no evidence that asking about suicide increases risk; there is substantial evidence that clinicians do not ask, that they ask indirectly when they do, and that the indirect question produces a denial.

The four sentences that are wrong, and the one that is right.

Said	Heard	Problem
"You are not thinking of doing anything silly, are you?"	"Say no"	"Silly" invites denial and insults the person
"Do you ever feel like you want to end it all?"	"That is a metaphor"	Vague; produces a vague answer
"You would not harm yourself, would you?"	"I am being watched"	Leading; the answer is in the question
"Are you having thoughts of suicide?"	"Yes or no"	Better, but a closed door
"Have you had thoughts of ending your own life — and if so, how often, and what have you thought about doing?"	"This person can hear it"	Correct: direct, specific, and open

Why family therapists get this wrong more often than individual therapists. Because in family work the therapist is managing multiple people's distress at once, and risk appears as one thread among many. The parent who says "she is being dramatic" and the adolescent who says "I am fine" produce a session that is busy, warm and dangerous. The discipline is structural: risk is assessed individually, in every case that presents with the features below, whatever the presenting complaint.

When to assess, as a minimum. Any presentation with: hopelessness, a recent loss, a previous attempt, a family history of suicide, a chronic illness or pain, substance misuse, a disclosure of self-harm, an adolescent with school refusal or sudden withdrawal, a client in a violent relationship, a client who has just been left, and any client about whom the therapist has a private unease she cannot name. The last is on the list because it is frequently correct.

Level	Picture	Action
Low	Ideation absent or fleeting, no plan, no intent, good support, reasons for living	Document, safety plan, review at next session, monitor

Level	Picture	Action
Moderate	Ideation with frequency, some intent, no plan or vague plan, inconsistent support, treatable	Safety plan, means restriction, notify a support person with consent, increase frequency, refer for assessment
High	Ideation with plan and intent, access to means, previous attempt, agitation, intoxication, no support	Do not leave the person alone; arrange urgent psychiatric assessment or emergency care; inform family; document contemporaneously

8.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The standard, and the four wrong sentences	Students write their own opening question, then compare
8–25	Five concepts: state not trait, the assessment is an intervention, means, family as resource and hazard, documentation	Lecture
25–45	The ten elements, each with its wording	Demonstration; then triads practise three elements each
45–55	Formulation and the three levels	Five vignettes, voted and defended
55–65	Self-harm: function, contagion, family response	Lecture with a case
65–78	The safety plan, written with the client	Live construction with a volunteer
78–90	The Suleiman case: an adolescent, a plan, and two parents	Case, students decide what the mother is told and when

8.2 Five concepts that organise the week

Concept 1 — Risk is a state, not a trait. The same person assessed in March and in July is two different patients. Risk rises with agitation, insomnia, intoxication, shame, loss of a relationship, and access to means, and falls with sleep, support and a reason to stay alive. The implication for practice is that an assessment is a snapshot with a date and a review, not a verdict filed once.

Concept 2 — The assessment is itself an intervention. A careful, direct, non-judgemental enquiry reduces isolation, communicates that the therapist can bear the material, and frequently produces the first honest conversation the person has had. This is why the quality of the asking matters clinically and not only forensically: a bad assessment does harm and a good one does some good.

Concept 3 — Means matter more than intent. The difference between a serious attempt and a death is often distance and method. Means restriction — locking medication, removing a firearm, agreeing that a friend holds the pesticide — is the intervention with the best evidence in the whole of this week's material, and it is the one most often omitted because it feels intrusive.

Concept 4 — The family is both the resource and the hazard. The family is the most reliable source of monitoring and the most common source of the precipitants: conflict,

shame, rejection, violence, and the loss of a relationship within the household. A family therapist is therefore unusually well placed to intervene and unusually at risk of talking the risk down.

Concept 5 — Documentation is a clinical act, not a defensive one. The record of what was asked, what was answered, how risk was formulated, who was informed and when it will be reviewed is what allows the next clinician to continue the work. It is also the only thing that distinguishes a considered decision from a negligent one when the outcome is bad, which it sometimes will be.

8.2a The ten elements, with wording

The elements are the standard list. What is usually missing from teaching is the sentence to use for each.

1. Ideation. "In the last two weeks, have you had thoughts of ending your own life?" Then: how often, how long does it last, can you stop it, does it come on its own or when something happens.

2. Plan. "Have you thought about how you would do it?" Then: where, when, have you prepared anything, have you been to the place. A plan that has been rehearsed is a different level of risk from a plan that has been imagined.

3. Intent. "Do you intend to act on this?" The question is uncomfortable and it is the one that most changes the formulation. A second question follows: "what has stopped you so far?"

4. Means. "Do you have access to — " and then name the means the client has already mentioned, plus the common ones in the setting: medication in the house, pesticides, a firearm, a height, a vehicle, a rope. Ask what is in the house and who else can reach it.

5. Previous attempts. "Have you ever tried before?" Then: what, when, what happened, did anyone know. The strongest single predictor, and frequently not in the referral.

6. Protective factors. "What keeps you going?" — children, a mother, faith, a dog, work, a plan for next year. Ask for specifics and record them in the client's words, because they are the raw material of the safety plan.

7. Substance use and impulsivity. "How much are you drinking?" and "do you do things quickly when you are upset?" Intoxication and impulsivity are the two factors that convert ideation into action within hours.

8. Family history. "Has anyone in your family died by suicide, or tried?" The question requires care and it must be asked; the answer frequently reframes the case.

9. Current stressors and recent losses. "What has happened in the last three months?" — a separation, a dismissal, a debt, a diagnosis, a death, an arrest, a public humiliation. Ask what the client expects to happen next, because the anticipation of a loss is as potent as the loss.

10. Disclosure and support network. "Who knows about this?" and "who could you call at two in the morning?" The answers determine whether a safety plan is possible or whether the plan must be a clinical arrangement.

The follow-up question students forget. "What would make it worse this week?" The answer is the plan: the anniversary, the court date, the visit from the husband, the results of the test. A risk formulation that does not name the date on which risk will rise is a formulation that will be wrong within seven days.

8.2b Formulation, and the sentence that carries it

An assessment produces a level, and the level produces actions. What makes the work defensible is the sentence that connects them.

The template. "Risk is assessed as [level] on [date]. This is based on [ideation, plan, intent, means, history]. Protective factors are [specific]. The precipitants in the next two weeks are [specific dates or events]. Actions taken: [means restriction, notification, referral, frequency of contact]. Review: [date]."

Worked example.

> Risk assessed as moderate on 14 March. Daily ideation over three weeks, no plan, no stated intent, prior overdose at 17. Access to a large quantity of her mother's medication in an unlocked cupboard. Protective factors: two children, a sister she telephones daily, church attendance. Precipitant: custody hearing on 26 March. Actions: medication moved to the sister's house today with the client's agreement; sister informed with consent; weekly sessions increased to twice weekly until the hearing; psychiatric assessment requested, appointment 19 March; safety plan written and copied. Review: 17 March.

Why this format works. It records the reasoning rather than the conclusion, so a reader can see that the assessment was made rather than assumed. It names the dates, so the review is real. And it records the client's consent to each notification, which is the detail that distinguishes a collaborative intervention from a unilateral one.

What must be added where the client refuses consent to notify. "Client declined notification of her sister. Discussed the additional risk of secrecy; client maintained refusal. Therapist notes the increased risk and will review at the next contact. Emergency contact details held." Recorded refusal is not misconduct; unrecorded refusal becomes, in a subsequent inquiry, an omission.

The three things a formulation must never contain. A promise — "client contracted not to harm herself" — which is worthless and which no client can make. A prediction — "client is not at risk" — which no clinician can make. And a euphemism — "client has some passive thoughts" — which conceals rather than records.

8.2c Ten things that raise risk in a family case

The formulation is built from the elements, and the elements are easy to gather and easy to leave unweighted. The ten items below are the ones that most often change a moderate assessment into a high one, and each is under-detected in family work because it appears as ordinary family material.

1. **A recent humiliation or exposure** — an arrest, a pregnancy discovered, a debt made public, a shaming in church or at work. The days immediately after an exposure are the days of highest risk.
2. **The end of a relationship in the last six weeks**, including the end that the client has not yet accepted.
3. **A court date, a hearing, or a custody decision in the next fortnight.** Ask for the date; it belongs in the formulation and it sets the review.
4. **Insomnia of more than three nights.** Agitation and sleeplessness are the states in which ideation becomes action.
5. **Access to a method that is unfamiliar to the therapist** — pesticides in a farming household, a relative's firearm, a well, a river the client has mentioned twice.
6. **A family history that nobody has asked about**, including a death that the family describes as an accident.
7. **A client who has become the family's carer** — for a parent with dementia, a disabled sibling, a dying spouse. The burden plus the loss of the role is a combination that appears in the files after the event.
8. **A client who has disclosed abuse and been disbelieved** by the family member she told.
9. **A client who is taking medication and has stopped it**, particularly where the family has discouraged it.
10. **The therapist's own unease, without a reason.** It is on the list because it is data: an experienced clinician's inarticulate concern is a signal that something in the presentation does not fit the formulation.

8.3 Self-harm: what it is, and what it is not

The distinction that matters clinically. Suicidal behaviour is intended to end life. Non-suicidal self-injury is intended to relieve intolerable feeling, to punish the self, to communicate, or to feel real, and is not intended to kill. The distinction is not absolute — self-injury is a risk factor for suicide, and the two occur in the same person — and it changes the response: self-injury requires an assessment of function and an alternative coping repertoire, and suicide requires a risk assessment and a safety plan.

The four functions, in the words clients use.

1. To feel something, or to feel nothing. "When I cut, everything goes quiet for a while."
2. To punish. "I deserved it." Frequently following abuse, and frequently accompanied by shame.

3. To communicate. "Nobody would listen unless they saw something." Common in adolescents in families where distress is not permitted.
4. To control. "It is the one thing that is mine." Common where the person's life is otherwise governed by others.

The countertransference. Self-injury produces strong reactions in therapists, and the reactions are usually one of three: rescue (the therapist becomes the person who must stop it, and the client's autonomy disappears), disgust (the therapist's own history or her culture's response to mutilation), or fascination (the case becomes the therapist's most interesting one). All three are material for supervision, and all three change the treatment.

What the therapist does in the session. Asks about the injury without recoiling; asks about the function without interpreting it; asks about the moment before, in detail, because the sequence feeling — act — relief — shame contains the points at which alternatives can be introduced; and does not make the stopping of self-injury a condition of the therapy. A therapist who says "you must stop" has added a rule to a problem that is caused by rules.

The family's response. The commonest sequence in a family: discovery, horror, prohibition, concealment. The adolescent learns to hide more carefully and to injure in places that do not show. The therapist's work with the parents is to convert horror into monitoring and prohibition into an agreement about what will happen when the urge arrives — usually a plan involving a person and a place rather than a rule.

Contagion. In school and peer settings, self-injury and suicidal behaviour cluster. Two implications. First, group work with adolescents who self-injure should be conducted with attention to comparison and to explicit method discussion; some services avoid mixed groups entirely. Second, in a family with two adolescents, the therapist should expect the second to be affected and should ask.

8.3a The safety plan, written with the client

A safety plan is not a no-suicide contract. It is a written sequence the client will follow as the urge rises, and it is written with the client, in her language, on paper she keeps.

The six steps, in order.

1. **My warning signs.** The thoughts, feelings, images and behaviours that come before the urge. In the client's words: "I stop answering messages", "I start thinking about my mother".
2. **Things I can do on my own.** Two or three, specific and available: a shower, walking to the junction, a particular playlist, prayer. Not "think positive".
3. **People and places that distract me.** Named people, with numbers, and places that are open at night.
4. **People I can ask for help.** Two or three names, with numbers, and an agreed script: "I am not all right, can you be with me for an hour?"

5. **Professionals and services.** The therapist's number with its limits, the clinic, the emergency department, the crisis line. Written in full, not abbreviated.
6. **Making the environment safe.** The means, and what will be done about them today: medication with the sister, the razor blades removed, the pesticide taken to the farm, the key to the roof given to the neighbour.

The rules for writing it. In the room, on paper, in the client's handwriting where possible; the client keeps a copy and the file keeps a copy; the client's own language, not the therapist's; reviewed at each session initially and at every change of circumstance; and the client's agreement sought at each step, because a plan imposed by the therapist is a plan the client will not use.

A worked plan, for Amina, 15, whose case follows in 8.5.

> **Warning signs:** I stop coming out of my room. I feel like I am behind glass. I start thinking nobody would notice. **On my own:** cold shower; the playlist Aunty Bisi made; walk to the corner and back. **Distraction:** Halima (mother) in the kitchen; sit with her while she cooks. Fatima next door. **Ask for help:** Halima — "I am having the bad thoughts again, please sit with me". Fatima — "can I come and sit at your place". **Professionals:** Dr Okafor (therapist) 0803..., weekdays 9–6; Amina's clinic 0805...; Emergency 112. **Safe environment:** mother's medication now in a locked box with the key kept by Halima's sister; Amina's blades given to the therapist in session; no access to the roof (padlock replaced 12 March).

8.4 Risk inside a family treatment

The structural rule. Assess individually, agree what will be shared, then facilitate the sharing. Three steps, in that order, every time. The therapist who assesses in front of the family gets a performance; the therapist who shares without agreement loses the young person; the therapist who neither assesses nor shares is the case in the file that ends badly.

Step 1 — individually. The client is seen alone, the assessment is conducted in full, and the safety plan is written. The parents or partner are told that the therapist is seeing the person alone and why, in general terms, before the session — never after.

Step 2 — agree what is shared. The conversation has three parts. What the therapist must tell, and why: the risk, because the household is the means of supervision. What the client will tell herself: the detail, the words, the timing. And what will not be told: the content of the therapy, the things said about the parents, the history. The agreement is written down, in the note and, where the client wishes, on paper for the family.

Step 3 — facilitate. The client speaks first, in the joint session, with the therapist present. If she cannot, the therapist says what was agreed and the client confirms it. The family's reaction is then worked with: the father's anger, the mother's guilt, the sibling's confusion. The session ends with the household's part of the safety plan — who checks the medication cupboard, who is called, what happens at night.

The three family configurations that require specific care.

The parent who is suicidal. The children are not told the content. They are told what changes: "your mother is ill and is being treated; she will be in hospital for a few days; this is not your fault and it is not because of anything you did." The children's need for a truthful, age-appropriate account is a clinical task and it is frequently skipped because the adults are overwhelmed.

The adolescent who self-injures. The parents are told the fact, not the diary. The prohibition response is anticipated and converted, in the session, into an agreement about the next occasion. The school is involved only with the young person's knowledge and, where the law requires it, with her being told first.

The child who discloses. The child's disclosure of abuse is Week 9's territory and the same structure applies: the child is seen alone, told what will happen next, and told who will be told and in what words before anyone is told.

8.4a Breaking confidentiality for risk, in words

The moment is unavoidable and it is where many therapists lose a client. The four-part statement, delivered before the call is made rather than after.

1. **The commitment, restated.** "I told you at the start that there are things I cannot keep, and this is one of them."
2. **The specific content.** "What I am going to tell your mother is that you have been thinking about taking her tablets, and that we have made a plan. I am not going to tell her what you said about her, or about your father, or anything about the sessions."
3. **The reason.** "Because the tablets are in the house, and because she can do something I cannot do from here, which is to be awake at two in the morning."
4. **The offer.** "You can be in the room when I tell her, or I can tell her and you can come in afterwards. Those are the two choices and I would rather you chose."

What the client usually says. "You promised." The answer is not a defence of the policy. It is: "I promised, and I told you at the time what the exception was. I am keeping the promise that matters more, which is that I would not let this happen without doing something."

What must never happen. Telling the family and then telling the client. The sequence converts a professional duty into a betrayal, and the client who learns of it afterwards will not disclose again — to this therapist or to any other.

8.4b When the family will not cooperate with the plan

The plan assumes a household that will act. Frequently it will not: the mother who says "she is doing it for attention", the father who refuses to lock anything because "a man's house is not a prison", the husband who dismisses his wife's ideation as manipulation.

The four responses, in order. First, ask what the objection is about, because it is usually about something specific: the father hears an accusation of his parenting, the mother hears that she has failed. Second, offer a version of the plan that does not require them to admit anything — locking a cupboard because the children are small, not because

a daughter is suicidal. Third, find another adult — an aunt, a teacher, a neighbour, a religious leader — and build the monitoring around that person, with the client's knowledge. Fourth, and where the risk is high, state the position plainly: "I am telling you what I think needs to happen because your daughter's life is at stake. If it does not happen, I will record that I asked and I will keep asking."

What not to do. Do not discharge the family because they will not comply. Do not accept the plan's collapse in silence. And do not turn the work into a campaign against the parents, which will cost the adolescent the only household she has.

8.5 Case: Amina Suleiman, fifteen

Referral. The school counsellor telephones: a teacher noticed cuts on the forearm of Amina Suleiman, 15, a senior secondary student. Amina has refused to explain them. The mother, Halima, 41, a trader, is telephoned and attends with Amina the following day. The father, Yusuf, 47, is a long-distance driver and is away; he will attend when he returns in three weeks.

The household. Halima and Yusuf, married nineteen years. Amina, 15; Ibrahim, 11. The family is Muslim; Halima's sister Bisi lives twenty minutes away. Yusuf is frequently absent for two to three weeks at a time.

Session one, with mother and daughter. Amina is silent, arms covered, answering in single words. Halima does the talking: Amina has been "difficult" since the new school year, has stopped eating with the family, and has been found crying in the bathroom. Halima's own account includes a period of depression two years earlier after a miscarriage, for which she took medication and did not continue.

The therapist does three things in this session and none of them is to ask about the cuts in front of the mother. She establishes the presenting problem in the family's terms. She explains the structure — she will see Amina alone first, then the family, and she explains why, in front of both, so that the individual session is not a secret. And she says what she will not keep: "if I think Amina is in danger, I will tell you, and I will tell her first that I am going to." Halima agrees. Amina looks up for the first time.

Session two, Amina alone, fifty minutes. For fifteen minutes Amina says nothing of substance. The therapist does not fill the silence with questions; she says, at minute twelve, "I am not going to make you show me your arm. I am going to ask you some questions that I ask everybody, and you can tell me to stop." Then the assessment.

- Ideation: yes, most days for about four months. "I think about not being here."
- Plan: yes. Her mother's remaining tablets, in the cupboard above the sink. She has taken them out twice and put them back.
- Intent: "I do not know. Sometimes I do." She has written a note, on her phone, unsent, four weeks ago.
- Means: the tablets, and the roof of the building, which has a door that is not locked.
- Previous attempts: none. Two episodes of cutting, both in the last three months, both on the forearm, both after arguments at home.

- Protective factors: Ibrahim, her brother — "he would find me"; her aunt Bisi; prayer; the possibility of finishing school.
- Substance use: none. Impulsivity: high — she describes decisions made "in one minute".
- Family history: none known; the therapist does not ask in detail and records that she will.
- Stressors: a change of school; her father's absences; an argument six weeks earlier in which Yusuf said she had "brought shame" by being seen with a boy; and the discovery that her mother had been "sick" and had hidden it.
- Network: nobody knows. Her closest friend has been told "some of it".

The formulation. High risk on the day of assessment: ideation with plan and some intent, means accessible in the house, a recent humiliating event at home, high impulsivity, and no adult who knows. Protective factors substantial: her brother, her aunt, her faith, school.

What the therapist did, in order, within that session.

1. She told Amina that she would be telling her mother, that evening, and exactly what she would say — that Amina has thoughts of ending her life, that there are tablets in the house, and that they need a plan. She did not tell her mother about the note, the boy, or the contents of the session.
2. She offered Amina the choice of being in the room. Amina chose to be present.
3. She telephoned the clinic's psychiatric service and made an appointment for two days later.
4. She wrote the safety plan with Amina (8.3a) and gave her a copy.
5. She asked Amina to hand over her blades, which Amina did, and recorded the receipt of them.
6. She saw Halima for twenty minutes, with Amina present, and used the four-part statement of 8.4a. Halima's response was silence, then weeping, then, "I did not know. I thought she was angry with me."
7. She asked Halima to move the medication that night and to lock the roof door; Halima telephoned her sister and the medication went to Bisi's house the same evening.
8. She wrote the note the same day: the ten elements, the level, the actions, the review date, and the consent recorded.

Sessions three to eight. The psychiatric assessment finds a moderate depressive episode without psychosis; no medication is started, and weekly therapy plus review is agreed. The family sessions, with Ibrahim present for two of them, work on the household's rules about distress: the family in which feelings were "not discussed", the father's shaming sentence, the mother's concealed illness, and the brother who had noticed everything and had been told nothing.

The father's return, session six. Yusuf attends, having been briefed by telephone by the therapist with Amina's knowledge. His first response is anger — at the school, at the boy,

at the therapist for "putting ideas in her head". The therapist does not defend herself. She asks one question: "what did you want for her?" He answers, "a good life, a reputation". The therapist says, "those are not the same thing, and I think one of them is killing her." It is the most direct sentence in the case, and it is delivered after five sessions of alliance-building and in the presence of the family. Yusuf is silent for a long time; then he says, "what do I do?"

Week twelve. Amina has not cut for nine weeks. The ideation has reduced to occasional, without plan. The family has a rule about the roof door and the medication, and Ibrahim knows what to do if his sister is "behind glass". Yusuf has changed his work pattern to be home one week in three.

What the case teaches.

1. **The assessment was done alone and the sharing was planned.** Had the therapist asked about the tablets in front of Halima, she would have received "I would never do that to my mother", and the plan would have remained in the cupboard.
2. **The disclosure to the mother was specific and limited.** Amina knew exactly what would be said. That is what made the second and third disclosures possible.
3. **Means restriction was the intervention.** Eight of the therapist's actions in that first session were administrative; the one that most reduced the risk was a locked box in another house.
4. **The family's culture was the terrain, not the obstacle.** Yusuf's concern for reputation was not contemptible; it was the value he had organised his life around. Naming it, and separating it from his daughter's life, was possible only after five sessions of being taken seriously.
5. **The mother's own history was in the case from session one.** Halima's untreated depression and her concealment of it were both the reason for Amina's silence and a risk in their own right. The therapist referred Halima for her own treatment in week seven.

8.6 Transcript: telling a fifteen-year-old that her mother will be told

Session two, minutes thirty-eight to forty-nine. Annotated.

> **A1 Therapist:** Amina, I have to tell you something and it is not a punishment. — The opening that distinguishes a duty from a consequence. Without it, everything that follows is heard as the therapist taking the mother's side. **A2 Amina:** You are going to tell her. **A3 Therapist:** Yes. (pause) Tonight, before you leave. — The fact confirmed immediately, with the timing. Delay in this conversation produces a fantasy of delay in the disclosure. **A4 Amina:** You said you would not. **A5 Therapist:** I said I would not tell her what we talk about. I also said that if I thought you were in danger I would tell her. (pause) You are in danger. The tablets are in the kitchen. — Both halves of the original agreement restated, in her words. A therapist who only restates the exception sounds as though she is invoking a technicality. **A6 Amina:** (crying) She will ask me why. **A7 Therapist:** She will. (pause) And you do not have to answer that tonight. Tonight is about the tablets. — The separation of the urgent from the important, which is what makes the disclosure bearable. The reason can wait; the means cannot. **A8 Amina:** She will cry and then she will be angry and then she will tell my father. **A9 Therapist:** She may tell your father. (pause) I am going to ask her not to telephone him tonight, and to let us decide together what he is told and by whom. I cannot promise she will agree. — An honest answer about a third party's behaviour, which is more reassuring than a promise the therapist cannot keep. **A10 Amina:** Can I be there? **A11 Therapist:** I was going to offer you that. (pause) You can be in the room, or you can wait outside and come in after I have said the first part. — The choice, given in the terms of 8.4a. The client's only remaining control is the manner, and it is enough. **A12 Amina:** I want to be there. But I do not want to talk. **A13 Therapist:** You do not have to talk. (pause) I will say the words, and you can nod. — The permission that makes her presence possible. A client who is required to speak in that room will not attend it. **A14 Amina:** (long pause) Are you going to tell her about the note? **A15 Therapist:** No. (pause) That is between us, and it is not what she needs tonight. — A limited, kept promise at the moment of breaking a larger one. The therapist's credibility rests on the second promise being exactly as reliable as the first.

What the transcript shows. Four of the seven therapist turns are about what will and will not be said, and none of them is a reassurance. The client's objections are answered with facts about procedure. The one thing the therapist does not do is apologise — "I am sorry, I have to" — which would present the disclosure as the therapist's misfortune and invite the client to console her.

8.7 Laboratory: the observed risk assessment

The exercise. Fifteen minutes with a trained actor, observed, pass or fail, and it counts towards the observed risk assessment component of the module. The actor is briefed with a presentation and a script that contains three opportunities to minimise and one moment of direct disclosure that must not be missed.

The six requirements. The student must (1) ask directly about ideation; (2) ask about plan, intent and means; (3) ask about previous attempts and protective factors; (4) ask what will make it worse in the next two weeks; (5) formulate a level aloud and justify it; (6) begin a safety plan with the client.

Criterion	Fail	Pass	Distinction
Directness	Euphemism, or never asked	Asked directly once	Asked directly, then followed up on frequency, controllability and content
Coverage	Fewer than six elements	Six to eight	All ten, with wording that produced specific answers
Means	Not asked	Asked	Asked, and a specific action agreed today
Formulation	None, or a label	A level with reasons	A level, a justification, a precipitate date and a review date
Safety plan	Suggested only	Begun	Written with the client, in her language, with means addressed
The relationship	Client defensive or silenced	Client engaged	Client discloses more at fifteen minutes than at five

The three most common failures observed. Asking the question once and accepting the first denial. Skipping means because the conversation has become emotional. And ending the assessment without a review date, which turns a snapshot into a verdict.

The debrief question. "What did you feel when she said she had thought about it?" Students report fear, relief, sadness, and a wish to reassure. Each of those is a signal of what the student will do under pressure, and each is worth naming before it is enacted.

8.7a Telephone risk, and the session that is not a session

A large proportion of risk work in a family practice happens on the telephone, and the telephone has its own method.

What to establish in the first four minutes. Where the client is, whether anyone is with her, whether she is safe for the next hour, and whether she has taken anything or is about to. Nothing else matters until those four answers exist.

What to do with the answers. If she has taken something, the call becomes a call for an ambulance and the therapist stays on the line, with the address written down and repeated aloud. If she has not, the call becomes a short assessment of the elements that can be assessed by voice — ideation, plan, intent, means, and who is nearby — followed by a decision: bring her in, send someone to her, or agree a contact in fifteen minutes.

The three things a telephone assessment cannot do. It cannot see the room, the tablets, the injury, or the other person in the house. It cannot establish whether the client is alone and says she is not. And it cannot end the call — the therapist who says "call me if you need me" and puts the phone down has handed the risk back at the moment it was highest.

The note. Written immediately after the call, before the next appointment, with the time of the call, the four opening answers, the action taken, and the time of the follow-up contact. Notes written the next morning lose the times, and the times are the evidence.

8.8 Six errors

Error 1 — The indirect question. "You are not thinking of doing anything silly?" The question is designed to be answered with a denial, by the therapist as much as by the client.

Error 2 — Assessing in front of the family. The parent says "she is being dramatic" and the assessment ends before it begins. Individual assessment is a structural requirement, not a preference.

Error 3 — The no-harm contract. "Will you promise me you will not do anything until we meet?" It has no evidentiary or clinical value, it shifts responsibility to a person who cannot carry it, and it has been followed by deaths in cases where it was recorded as protective.

Error 4 — Means left in the house. The assessment identifies the tablets and the plan records them, and nothing is done about them. This is the single most common omission in the files of completed suicides.

Error 5 — Telling the family before telling the client. The disclosure is made in a telephone call "for safety" and the client learns of it afterwards. The relationship ends, and the next clinician receives a client who will not disclose.

Error 6 — Treating self-harm as a behaviour to be stopped. The prohibition produces concealment, the concealment removes the client from view, and the risk rises while the file says the behaviour has stopped.

8.9 Culture, faith and suicide

The religious frame is not an obstacle to the assessment; it is material within it. In households where suicide is understood as a sin and a shame upon the family, the client's ideation will be concealed with particular skill, and the protective factors will include the very belief that produces the shame. The therapist asks about both: what the client believes would happen to her soul, and what she believes would happen to her mother. The answers frequently reveal that the religious prohibition is holding the person alive and that the social consequence is pushing her toward death, at the same time.

Confession and concealment. A client who has confessed suicidal thoughts to a religious authority and been told to pray more has been given a penance without an assessment. The therapist's response is not to criticise the authority but to make the assessment: "what did you tell him, and what did he say, and did anyone ask you whether you had a plan?"

The family's reputation after a death. Where a death by suicide carries lasting stigma, the family's grief work includes managing the community's account of the death. This is a real clinical task and it is regularly ignored by models of bereavement developed elsewhere. The therapist helps the family decide what will be said, to whom, and by whom, and treats the family's need for a survivable story as legitimate.

Postvention. After a death by suicide, the family's risk rises: bereaved relatives have elevated rates of suicide, and the risk is highest in the first year and among parents and

partners. The therapist's postvention work with a family includes assessing each member, naming the risk explicitly with the adults, and agreeing who will watch whom in the first three months. It is the case in which the therapist is most likely to be invited to comfort and least likely to assess.

The therapist's own exposure. A death by suicide in a therapist's caseload is an occupational hazard of clinical work and it produces a characteristic response: a review of every session, a conviction of responsibility, and either an increase in defensive practice or a withdrawal from risk entirely. The professional response is a structured review with a supervisor, a written account of the care provided, and support. The unprofessional response is silence, which converts one death into a pattern of avoidance that affects every subsequent client.

> **DEBATE**
> "When a competent adolescent refuses to allow her parents to be told about her suicidal thoughts, the therapist should respect her refusal." Argue for: the young person's developing autonomy, the evidence that forced disclosure ends engagement, and the clinical value of being the one adult who does not betray her. Argue against: the household is the only available supervision, the means are in the house, and the law in most jurisdictions permits or requires parental involvement. Then state the threshold at which you would override the young person, and who you would tell first.

8.9a The therapist's own history of suicide

Many clinicians have lost someone to suicide, have survived an attempt themselves, or both, and the training literature treats this as though it did not exist.

The rule. A therapist with an unprocessed personal history in this area is at risk of two opposite errors: the under-assessment that comes from familiarity ("she sounds like I did, and I am fine"), and the over-reaction that comes from fear ("I cannot take a client with ideation at all"). Both are clinical problems and neither is a character flaw.

The questions to ask yourself, in writing, once. Whose death or attempt is nearest to this case? What do I do when a client describes a method I have thought about? Am I assessing this client or managing my own fear? The answers determine whether the work needs consultation, a change of caseload, or personal therapy.

What is not acceptable. Practising without supervision in this domain while carrying an unexamined history, and disclosing the history to the client in a way that makes her responsible for the therapist's distress. "I lost my own brother, so I understand" is occasionally the most useful sentence in a session and is more often the moment at which the client stops being the patient.

8.10 Evidence note

What the research supports. Asking about suicide does not increase risk, and screening improves detection. Previous attempt is the strongest single predictor. Means restriction reduces suicide rates — the evidence from pesticide regulation, firearm access and medication packaging is among the strongest in the field. Safety planning interventions reduce subsequent suicidal behaviour relative to usual care, and brief contact interventions after discharge reduce repetition. The therapeutic alliance is associated with reduced suicidal ideation in treatment studies.

What is weaker. Risk assessment tools have modest predictive value at the individual level and should not be used as a substitute for clinical formulation; the field has largely accepted that prediction is not achievable and that formulation, safety planning and continuity of care are the useful targets. The evidence base for no-harm contracts is essentially null and their use is discouraged.

What is not supported. That hospitalisation prevents suicide in every high-risk case — the post-discharge period is the period of highest risk. That a client who denies ideation is safe. That a low score on an instrument settles the question.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same disclosure of ideation in a week when the client's mother is away and the tablets are unlocked, and in a week when the household is stable, are two different risks.

8.11 Key terms

Ideation — thoughts of ending one's life; assessed for frequency, intensity, controllability and content.

Plan — a specified method, place or time; rehearsed plans indicate higher risk.

Means restriction — reducing access to lethal methods; the intervention with the strongest evidence base.

Safety plan — a written, client-authored sequence of steps from warning sign to emergency contact.

Non-suicidal self-injury — self-harm without intent to die, performed for relief, punishment, communication or control.

Postvention — the support and risk management provided after a death by suicide.

Formulation — the clinical reasoning that links the assessment to a level of risk and a set of actions.

8.11a Three sentences that are remembered

The sentence that opens the assessment. "I ask everybody this, because it matters and because people are usually relieved to be asked: have you had thoughts of ending your own life?" The framing does two things — it normalises the question and it names the relief, which is usually accurate.

The sentence that survives a denial. "All right. And if it came back, who would you tell?" A denial is information, not a conclusion, and the follow-up question keeps the door open without an interrogation.

The sentence that ends a session safely. "Before you go: what is the plan for tonight, and who is in the house with you?" Asked at the door, it is heard as care rather than as assessment, and it produces information that the earlier questions did not.

8.12 Self-test

1. Give the correct wording for asking about suicidal ideation, and explain why the four common versions fail.
2. State the ten elements of the assessment, and the follow-up question most often forgotten.
3. What are the actions attached to each of the three levels?
4. Why is a no-harm contract not a safety plan?
5. Give the six steps of a safety plan.
6. In the Suleiman case, what four actions most reduced the risk, and why?
7. What is non-suicidal self-injury, and what are its four functions?
8. What is postvention, and why is the family at elevated risk?

Answers. (1) "Have you had thoughts of ending your own life — and if so, how often, and what have you thought about doing?" The common versions use euphemism, vagueness, leading phrasing, or a closed question, each of which invites denial or produces an unusable answer. (2) Ideation, plan, intent, means, previous attempts, protective factors, substance use and impulsivity, family history, current stressors and recent losses, disclosure and support network; the forgotten question is what will make it worse in the next two weeks. (3) Low — document, plan, review; moderate — plan, means restriction, notify with consent, increase frequency, refer; high — do not leave the person alone, urgent psychiatric or emergency care, inform family, document contemporaneously. (4) Because a contract extracts a promise from someone who cannot guarantee it and produces no actions, whereas a plan specifies who does what, when, with what means removed. (5) Warning signs; things I can do alone; people and places that distract; people I can ask for help; professionals and services; making the environment safe. (6) The individual assessment, which produced the information; telling the mother with Amina's knowledge and a limited content; the removal of the medication to another house; and the locked roof door — the last two being means restriction, the intervention with the strongest evidence. (7) Self-harm without intent to die, performed to relieve intolerable feeling, to punish, to communicate, or to control. (8) The support and risk management of those bereaved by suicide; because bereaved relatives, particularly parents and partners, have elevated suicide rates, highest in the first year.

8.13 Reflective task

Write the note you would want a coroner to read about a client you were worried about. Then write the note you actually keep. The difference between the two is the measure of your documentation practice.

8.14 Further reading

- Stanley, B. and Brown, G. on safety planning — the source of the six-step structure used here.
- Jobes, D. on the collaborative assessment and management of suicidality — the best-developed clinical approach to formulation and alliance.
- Hawton, K. and colleagues on self-harm in adolescents — the epidemiology and the evidence on repetition.
- The WHO suicide prevention materials, particularly on means restriction at population level.
- Any current national guideline on the assessment and management of suicide risk in your jurisdiction.

Next: Week 9 — Risk II: intimate partner violence and child protection.

Week 9 — Risk II: intimate partner violence and child protection

Learning outcomes. By the end of this week the student can (1) distinguish situational couple violence from coercive control and explain why the distinction changes the treatment; (2) screen each partner separately with direct questions, and document a positive screen; (3) identify the factors associated with lethal risk and act on them; (4) explain to a couple why conjoint work is contraindicated and what is offered instead; (5) build a safety plan with a victim that does not require her to leave; and (6) discharge the statutory duty to report a child protection concern correctly and in the right order.

The distinction on which everything depends

	Situational couple violence	Coercive control
Pattern	Mutual, low-level, escalates under stress; both partners can be aggressive	One partner dominates systematically; the violence is one instrument among many
Fear	Situational, episodic	Chronic; the victim is afraid most of the time
Mechanisms	Arguments that escalate	Surveillance, isolation, financial control, threats, sexual coercion, rules, humiliation
The victim's view	"We both lose control"	"I have to be careful all the time"
Couple therapy	May be appropriate after assessment	Contraindicated; risk increases after conjoint sessions
Response	Communication and de-escalation work; individual safety plan	Safety planning, advocacy, protection; individual work only

Why the distinction is not academic. Conjoint work in a coercive control case does three harmful things. It treats the violence as a communication problem, which the perpetrator welcomes. It gives the perpetrator access to the victim's disclosures, because everything said in the room is available to him afterwards. And it produces retaliation: the literature and the practice literature agree that the period after a conjoint session is a period of elevated risk, and the retaliation is frequently about something said in the session.

The family therapist's structural problem. Couple and family therapy is the modality most often requested by couples in which control is present, because the request itself is part of the control: "we need help communicating" is a formulation in which the violence does not appear. The therapist who accepts the referral at face value will conduct twelve sessions of communication training in a household where one partner is deciding what the other may say.

9.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	The two patterns; how the referral conceals the second	Two referral letters, students identify which is which
10–25	Five concepts: control, lethality, separation, the structural problem, the child as victim	Lecture
25–40	Screening: who, when, how, and what to do with a positive	Demonstration, then paired practice of the four questions
40–55	The lethality factors, in plain language	Card sort; each student ranks five cases
55–70	Ending conjoint work, and what to offer instead	Role-play of the conversation with a couple
70–80	The reporting duty: threshold, route, order of operations	Local rules quiz
80–90	The Okafor case	Case, with students deciding what is reported and to whom

9.2 Five concepts that organise the week

Concept 1 — Coercive control is a pattern of domination, not a series of incidents. The incidents are the visible part; the substance is the regulation of the other person's daily life — what she wears, whom she sees, whether she works, whether she has money, whether her phone is checked. The clinical implication is that an absence of recent violence is not an absence of danger, and that the therapist must ask about control, not only about hitting.

Concept 2 — Lethality is assessable, and the factors are specific. Strangulation, threats to kill, access to a weapon, stalking, separation, pregnancy, escalation in frequency or severity, a history of violence towards others, and a partner who has been violent during sex. Each raises the risk of homicide substantially. The therapist does not need a scored instrument to ask about them, and should know the local one if it exists.

Concept 3 — Separation is the period of highest danger. The weeks after a woman leaves, or after she says she will, are the weeks in which the risk of homicide peaks. This inverts the assumption of many helpers, who treat leaving as the solution; leaving is a process with a peak, and the safety planning must cover the leaving rather than begin after it.

Concept 4 — The therapy setting is a site of risk, not a sanctuary. Appointment times are known, waiting rooms are public, text messages are visible, and a partner who is controlling will want to attend every session. The practice must be designed with this in mind: separate contact details, notes that cannot be read at home, a protocol for what to say if the partner telephones, and a receptionist who has been trained.

Concept 5 — The child is a victim of the control, not only a witness. Exposure to the control of one parent by another is a form of harm in its own right, recognised in child protection frameworks in most jurisdictions. The child's presence changes the therapist's duties, the risk formulation and the plan.

9.2a Screening: who, when, how

Who. Every adult seen in a couple or family case, at least once, alone. Not "where indicated", because indication is what the screening is for.

When. In the first two sessions, before any conjoint work, and again if the presentation changes — a new injury, a new fear, a partner who has become more attentive to the therapy.

How. Alone, in a room the partner cannot see into, with an explanation of why the question is being asked. The four questions, in this order:

1. "Has your partner ever hit, kicked, pushed, slapped or otherwise hurt you?"
2. "Has your partner ever made you afraid — not angry, afraid?"
3. "Has your partner ever forced you to do something sexual that you did not want?"
4. "Do you feel safe going home today?"

The follow-ups that produce the information.

- "What is the worst thing he has ever done?" The answer is frequently not in the first four questions.
- "Has he ever put his hands around your neck, or made you unable to breathe?" Strangulation is under-reported and is the single strongest non-fatal predictor of homicide.
- "Has he ever said he would kill you, or the children?"
- "Does he check your phone, your money, where you go?"
- "Has the violence got more frequent or worse in the last year?"
- "What happens after he has been violent — does he apologise, blame you, promise, or nothing?"

What to do with a positive screen. Thank the client, name what she has described, and ask what she wants. Then the sequence: assess lethality; agree immediate safety for today; discuss what the therapist must do (children, statutory duties); provide information about services without making attendance a condition; record; and plan the next contact. The full safety-planning method is in 9.3a.

What not to do. Do not ask in front of the partner. Do not ask as part of a checklist read aloud to both. Do not respond with "have you considered leaving?" as the first sentence, which is heard as "your judgement has been poor". And do not offer couple therapy as the response to a positive screen.

9.2b The lethality factors, in the language of a session

The list below is the one a family therapist should be able to run through without notes. None of them is a prediction; each of them changes the urgency.

1. Strangulation. Ask: "has he ever put his hands around your neck, or covered your mouth and nose?" A woman who has been strangled is at substantially elevated risk of being killed, and the assault frequently leaves no visible injury and is not reported to the police.

2. Threats to kill. Ask for the words. "I will kill you and nobody will find you" is a different fact from "I was angry".

3. Access to a weapon. A firearm, a machete, a knife kept in the bedroom. Ask what is in the house.

4. Separation, or the threat of it. Ask: "have you told him you want to leave, or has he said he would not allow it?" The answer sets the risk of the next month.

5. Stalking and monitoring. Following, checking the phone, tracking the car, arriving unannounced, calling the therapist. Monitoring is a marker of the partner's need for control and of his capacity to find her.

6. Escalation. More frequent, more severe, or new forms. Ask for a comparison with a year ago.

7. Pregnancy and the postnatal period. Violence frequently begins or escalates in pregnancy, and the risk of homicide is elevated.

8. Sexual violence within the relationship. Ask directly; the question is rarely asked and frequently answered.

9. A history of violence outside the home — towards previous partners, children, strangers, animals.

10. The partner's own instability — substance misuse, a recent job loss, a mental illness, a court case, jealousy that has become fixed. Combined with a weapon and a separation, this is the configuration that appears in the files after a death.

How to use the list in a session. Not as an interrogation. Three or four questions, chosen for what the client has already said, in a tone that says "these are questions I ask

everybody". The client who is asked about strangulation by a therapist who is not shocked will answer, and the answer may be the first time she has said it aloud.

9.2c The practice that is safe to be a client in

Safety is a property of the practice, not only of the therapist's intentions. Six arrangements, each cheap and each routinely absent.

1. **A second contact number that the partner does not know.** A client's contact details must be recorded with a note on who may be telephoned and what may be said. The receptionist who telephones a house and asks for "Mrs Okafor about her appointment" has disclosed the therapy.
2. **A receptionist who has been briefed.** The person at the front desk is part of the clinical service. She needs two sentences: do not confirm whether a person is a client; if a man arrives asking for his wife, say you will check and get the therapist.
3. **A waiting room that is not a corridor.** Where a couple is in a violent relationship, the waiting room is where the confrontation happens. Separate entrances, staggered appointment times, or the client waiting in the car and being fetched.
4. **A protocol for the partner's telephone call.** Written down: what may be said, who says it, and the note that is made afterwards. Usually: "I can confirm that she had an appointment and I cannot discuss anything else."
5. **Notes that cannot be read at home.** If the client takes a receipt, an appointment card or a leaflet home, it will be found. The practice's answer: nothing written goes home unless the client asks for it.
6. **A named person to call at night.** Not the therapist's personal number as a general service, and not a promise of availability. A named duty line, written on a card, with the hours stated.

The test. Ask the receptionist, or the answering service, what she would say if a man telephoned and said, "my wife comes to you, I want to know when her appointment is". If the answer is not rehearsed, the practice is not ready for this week's cases.

9.3 Ending conjoint work: the conversation and the alternative

The therapist has screened both partners, and the screen is positive. The couple is expecting a session about communication. The task is to change the treatment without losing the victim.

The principles. Do not confront the perpetrator with the accusation in front of the victim, because she will pay for it. Do not tell the victim's account to the perpetrator. Do not use the word "violence" unless the victim has used it in front of him, because the disclosure of what she said in the individual session may itself be dangerous.

The script, with both present.

1. "I have met each of you separately, as I said I would, and I have decided that the work I do — seeing couples together — is not the right thing for you at the moment."

2. "The reason is that I do not think this is a communication problem, and couple work can make things worse when it is not."
3. "Here is what I am offering instead: individual sessions for each of you, with me or with a colleague, and a referral for [the relevant service]."
4. "I want to be clear about one thing: I will not be discussing with either of you what the other has said to me."

The predictable responses, and the answers.

- The perpetrator's anger: "why does she get her own sessions?" — "Because I need to see each of you alone to do this work, and that is the same for both of you." (The symmetry is real and is stated without disclosing anything.)
- The victim's fear: she may not want to end conjoint work, because the conjoint sessions are the only place she is safe from him. This is a genuine clinical dilemma. The resolution is not to continue conjoint sessions but to increase the frequency and reliability of individual contact, and to help her build other safe contacts.
- The couple's refusal: "we came for couple therapy". Then the therapist states the limit as a limit on her own practice: "I am not able to do the work you are asking for, because I think it would put one of you at risk. I would rather lose the case than do that."

What is offered instead, and why each element matters.

Offer	Purpose
Individual sessions with the same therapist, for the victim	Continuity and a safe place to plan
Individual work with a different therapist for the perpetrator	Avoids the therapist holding both accounts and being triangulated
Referral to specialist advocacy or a domestic violence service	Practical help the therapist cannot provide: housing, police, legal, money
A referral to a perpetrators' programme where one exists	The only intervention with evidence of effect, and only where attendance is monitored
A child protection referral where children are present	The statutory duty, discharged separately from the couple's treatment

The therapist who continues couple work anyway. The commonest reason is the therapist's belief in the relationship or in her own skill. The second commonest is the victim's request. Neither is a justification, and the note written after an injury will not read well: "continued conjoint work despite disclosure of coercive control in individual session of 4 March."

9.3a Safety planning with a victim

The framing sentence. "I am not going to tell you to leave. Whether you leave is your decision and you know your situation better than I do. What I want to do is make sure that if you ever need to leave quickly, you can."

That sentence is the foundation, because the victim has usually been told by professionals, relatives and her husband what to do, and the therapist who does not add to the list becomes usable.

The plan, in eight parts.

1. **The documents.** Identity card, birth certificates, the children's records, the marriage certificate, bank cards, a copy of any protection order. Kept in a place he does not know, or with a trusted person.
2. **The money.** A small amount saved, a separate account if possible, and knowledge of where to get emergency assistance.
3. **The bag.** A packed bag at a neighbour's or a sister's house: clothes for the children, medication, a phone charger, a copy of documents.
4. **The code word.** One word, texted to one person, meaning "come now" or "call the police". Agreed in advance, with a person who will act.
5. **The route.** Where she goes, how she gets there, and what she does if the first place is not available. Two options, not one.
6. **The children.** Where they are at each hour of the day, who collects them, and what the school will do if he arrives.
7. **The phone.** A second phone or a saved number; the emergency number on speed dial; and the practice's policy about what the therapist will say if he answers her phone.
8. **The review.** The plan is reviewed at each contact, because circumstances change and because the plan that was made in March may be wrong in June.

What the therapist does not do. Does not hold the bag, the documents or the money. Does not promise to be reachable at all hours. Does not contact the perpetrator or mediate. Does not tell the family, the church, or the employer without consent, except where a child protection duty requires it.

The danger of the written plan. A safety plan found in a handbag or on a phone can escalate the violence. The practical solution is that the victim keeps the information in her head or in a form that can be explained away, and the therapist keeps the record; the copy given to the client is minimised and, where necessary, not given at all.

9.4 Child protection: recognition, threshold, duty

The four categories. Physical abuse; sexual abuse; emotional abuse; neglect. In most jurisdictions the definition also includes exposure to domestic violence and, increasingly, exposure to harmful practices. The therapist's task is not to determine whether abuse occurred — that is the job of the investigative authority — but to recognise a concern at the threshold the law sets, and to report it.

The threshold in plain language. A concern is reportable when there is a reasonable suspicion that a child is being harmed or is at risk of significant harm. Not proof; suspicion. The therapist does not need to be right; she needs to be reasonable, and the reasonableness is judged on what she knew and when, not on the eventual outcome.

What raises a concern in ordinary family work.

- A disclosure by a child, however tentative or retracted.
- An injury with an inconsistent explanation, or an explanation that changes.
- Sexualised behaviour in a young child that is developmentally unexpected.
- A child who is frightened of a specific adult, or who will not go home.
- A parent's account of violence towards a child, including "discipline" that leaves injury.
- Exposure to the coercive control described in 9.2 — a child who sees it, hears it, or is used in it.
- Chronic neglect: hunger, untreated illness, absence from school, a child left with inadequate care.
- A child who discloses that an adult has asked him to keep a secret that frightens him.

The reporting route in Nigeria, and the general principle. The Child Rights Act 2003 has been adopted by many but not all states, and state child protection laws vary; the practical route is usually the state ministry responsible for social welfare or the state child protection unit, the police family support unit, or a designated safeguarding lead in a school or hospital. The therapist should have the four local numbers in her practice manual, verified within the past year, and should know which of them operates at night.

The order of operations, which matters more than the route.

1. **Ensure immediate safety.** If the child is in danger now, the emergency services come first and the paperwork second.
2. **Record what was said, in the child's words,** before speaking to anyone else, with the date and time.
3. **Consult** — a supervisor, a designated safeguarding lead, a child protection adviser — where consultation is available within the hour. Consultation does not delay a report that is required.
4. **Report.** The call is made by the therapist, recorded with the time, the name of the person spoken to and the advice received.
5. **Tell the parents, unless telling them would endanger the child.** The default is transparency, and the exception is where a parent is the alleged abuser or the parent's response would expose the child to further harm or to pressure to retract. The decision is recorded with reasons.
6. **Tell the child what will happen,** in words she can hold: who will be spoken to, what she will be asked, and that she has not done anything wrong.
7. **Continue the treatment if possible,** and record the referral and the outcome.

What the therapist does not do. Does not investigate. Does not interview the child repeatedly to establish the facts, which contaminates the evidential process and harms the child. Does not confront the alleged abuser. Does not promise the child confidentiality. And does not accept the family's assurance that "it will not happen again" as a reason not to report.

9.4a The child's disclosure: what to say, and what not to say

What to say. Four things, in this order. "Thank you for telling me." "You have not done anything wrong." "What you have told me is important and I am going to have to tell someone who can help — I will tell you who and what I am going to say before I do it." "I am not going to ask you to tell the story again and again."

What not to say. "Are you sure?" "Why did you not tell me before?" "Did he touch you here?" — the leading question that puts the words in the child's mouth. "Promise me you will tell me if it happens again." And "this will be our secret", which is the sentence that makes the child complicit and which the therapist of all people must not say.

Recording. Verbatim, in quotation marks, including the child's own vocabulary and the questions that were asked. "She said 'he does a bad thing to me at night'. I asked 'can you tell me what happens?' She said 'he comes to my bed and I pretend to sleep'." That record is markedly more useful than "child disclosed abuse".

The child's retraction. Common, and frequently the product of pressure at home. A retraction is recorded, reported and not treated as a resolution. The therapist's note: "child retracted the account on 12 April, having spent the weekend with the maternal grandmother; the original disclosure stands recorded and the referral continues."

The non-abusing parent. Frequently the person who brings the child, frequently disbelieving, frequently the target of pressure from the extended family to "settle it". The therapist's work with her is specific: what she believes, what she has been told by whom, what she is afraid of, and what she will do if the child is disbelieved by the family. Her capacity to protect is the single best predictor of the child's outcome, and supporting it is the most useful clinical act available.

9.4b When the report is made against the family's wishes

The family will be angry, and the anger is understandable: they came for help and received an investigation.

What the therapist says. "I know this is not what you wanted. I am required to report what I am told when a child may be at risk, and I have done it. I am telling you now, before anyone contacts you, because I do not want you to be surprised." Then the specifics: who was called, what was said, what is likely to happen next.

What the therapist does not apologise for. The report itself. An apology — "I am so sorry, I had no choice" — converts a professional duty into a personal betrayal and invites the family to treat the therapist as the source of the problem. The better sentence: "I did what the law requires, and I am still your therapist if you want me to be."

When the family leaves. A proportion will. The therapist records the reasoning, offers the referral, and does not pursue. A family that leaves because a report was made may return, and the file should make that possible: a closing summary, a note that the door is open, and no editorial about the family's decision.

Where the alleged abuser is a client. The therapist may be treating the perpetrator — frequently a father in a family case — and now has a duty to the child that conflicts with

the duty to the client. The resolution is not subtle: the child's safety governs. The therapist reports, tells the client that she has done so and why, and considers whether she can continue to treat him. In many cases she cannot, and the transfer is the correct outcome.

9.4c The four cases that must not be missed

Four presentations in which the child protection duty is engaged and in which family therapists most often hesitate.

The one that is "discipline". A father describes beating his nine-year-old with a cable "because he is stubborn". He is not asking for help with it; he is describing ordinary parenting. The therapist's duty is engaged if the law's threshold is met, and the conversation is not about whether the father loves his son. The therapist says what she must do, and does it.

The one that is "cultural". A family requests a practice that is harmful to a child — a cutting, a beating as initiation, a marriage of a fifteen-year-old. The therapist does not adjudicate the culture. She applies the law, states it plainly, reports where required, and works with whatever part of the family will work with her.

The one where the child is used. A mother reports that her husband's family have taken her son to another state and will not return him. This may be abduction, custody dispute, or child protection matter; the therapist does not decide which, and she does telephone the authorities and record the advice.

The one that is a disclosure by an adult. A client in individual therapy says, in passing, that her brother "does things" to her eight-year-old niece. The duty is engaged for the niece, who is not a client and has never been seen. The therapist reports, tells the client that she is doing so, and works with the client's guilt afterwards.

The common thread. In each of the four, the therapist's hesitation is about the relationship with the adults. The child is not in the room. That is exactly why the duty exists, and why the decision cannot be left to the clinical judgement of the person who is fond of the family.

9.5 Case: the Okafors, and the referral that said "communication"

Referral. Blessing Okafor, 34, a hairdresser, telephones a couple practice: "we need help with communication. My husband has agreed to come." Emeka Okafor, 41, a spare-parts trader, attends the first session. Children: Chidi, 6, and Ada, 2.

Session one, both present. The complaint is that "she is always angry and she shouts in front of the children", and "he does not listen". The therapist, following the practice protocol, explains that she will see each of them alone once before beginning conjoint work, and gives the reason: it is how she gets to know each person. Emeka objects mildly, is reassured, and agrees.

Session two, Blessing alone, and what she discloses.

- Physical violence for nine years, beginning in the first year of marriage. Pushing, slapping, a beating during her second pregnancy that led to a threatened miscarriage.

- **Strangulation**, twice, the most recent eight months ago.
- Threats: "he says he will kill me and take the children to his village, and nobody will ask him any questions."
- Control: he keeps her bank card, checks her phone every night, decides whether she may visit her mother, and has twice arrived at her salon and sat outside.
- Sexual coercion: "sometimes I let him so that he will not be angry."
- Isolation: her mother lives in another state; her closest friend has stopped visiting because of him.
- Threats about the children: "he says if I leave he will make sure I never see them."

The assessment. Coercive control with high lethality indicators: strangulation, a specific threat to kill, a threat to remove the children, monitoring of her movements, escalation during pregnancy, and recent escalation in frequency. There is also exposure of the children to the violence: Chidi has been present at four incidents and has begun wetting the bed.

The therapist's actions in that session, in order.

1. She did not ask "why have you not left". She said: "thank you for telling me. None of this is your fault."
2. She assessed immediate safety: "are you safe going home today?" Blessing said she was, "as long as I do not annoy him".
3. She asked about the children and made the note that establishes the child protection question: present at four incidents, bedwetting since the last one.
4. She explained what she must do. She would not be reporting to the police — that is the client's decision — but she was obliged to report the children's exposure to the state child protection unit, and she would tell Blessing exactly what she would say and when. She was clear that the report concerned the children, not the marriage.
5. She made the report the same afternoon, after a supervision call, and recorded the name of the officer and the advice.
6. She arranged a second individual session in three days, and gave Blessing the practice's emergency number and a code phrase — "I am calling about the hair appointment" — that would tell the therapist that she needed to talk and could not speak freely.
7. She documented the safety plan in the 9.3a form, with the documents left with Blessing's aunt and a bag at a neighbour's house.

Session three — the conjoint session that did not happen. Emeka attends expecting the couple session and is told, with Blessing present, that the therapist will not be conducting conjoint work. The therapist used the script of 9.3, and Emeka's response was as predicted: anger, then a demand to know what his wife had said, then a threat to withdraw from the process altogether.

The therapist's answer, which is the one that matters. "I am not going to tell you what she said. What I will tell you is my clinical view, which is that the two of you working on

communication in this room would not be safe, and I am not willing to do it. I will see you individually, and I will give you the name of a programme that works with men in your situation."

Emeka did not attend individual sessions. Blessing attended eleven over four months.

The child protection process. A social worker visited the home twice. No removal. A case was opened and the children were seen at school. Blessing was asked, by the social worker, whether she wanted to leave; she said she did not, yet. The therapist supported her in that answer in the recorded conversation with the social worker, which is the point at which many therapists instead advocate for the outcome they believe in.

Month five — the escalation the plan anticipated. Blessing telephoned at 9.40 p.m. from her sister's house. He had found the bag at the neighbour's, had been "calm", and then had thrown a chair through the window of the room where Chidi was sleeping. She had left with both children. The therapist's role in that call was three things: confirm immediate safety, direct her to the police station and the social worker, and be on the telephone the following morning when she made the statement. She did not go to the house, did not confront Emeka, did not offer her own home.

The outcome, which is not a happy ending and is a good outcome. Blessing did not return. A protection order was applied for with the help of a legal aid organisation. Emeka was charged and the case is before a court. Blessing attended therapy for a further eight months, and the children were seen by a child therapist. The therapist's own note after the night call reads, in part: "the plan made in session two covered the leaving. It did not cover what he would do when he found the bag. The bag should have been somewhere he had no reason to go."

What the case teaches.

1. **The referral said "communication" and the case was coercive control.** The screening was the only thing that found it, and the screening was a practice protocol rather than a clinical hunch.
2. **The individual session was the instrument of safety, and the conjoint session was the instrument of danger.** Everything the case required came out of the individual sessions; everything that would have been harmful would have happened in the joint one.
3. **The child protection report was separate from the marriage.** It concerned the children's exposure, it was made with the mother's knowledge, and it did not require her to leave or to make a statement against her husband.
4. **The therapist did not tell her to leave, and she left.** The decision was hers, made five months later, and it was made because the plan and the relationship were in place.
5. **The plan had a hole, and the therapist recorded it.** The bag's location was a foreseeable mistake, the note says so, and that note is what a supervisor needs in order to improve the practice's protocol.

9.6 Transcript: "I am not going to tell you what she said"

Session three, minutes six to nineteen, with both present. Annotated.

> **B1 Emeka:** So what did she tell you? Because she has been telling people things. **B2 Therapist:** I am not going to tell you what she said. (pause) I am going to tell you what I have decided. — The refusal and the substitution, in one breath. The refusal alone leaves a vacuum that he will fill with suspicion; the substitution gives the session a subject. **B3 Emeka:** So you have decided something without me. **B4 Therapist:** I have. (pause) I have decided that I will not see the two of you together. — Ownership of the decision. The therapist does not say "the practice's policy requires", which would be true and would invite him to argue with an absent authority. **B5 Emeka:** Why? **B6 Therapist:** Because I do not think this is a communication problem, and because couple work in a situation like yours can make things more dangerous afterwards. — "A situation like yours" rather than "your violence": the therapist has not disclosed the individual session's content and has not accused him, and the meaning is unmistakable. **B7 Emeka:** (to Blessing) You told her I hit you. **B8 Therapist:** (to Emeka, immediately) I am talking to you now, not to her. — The intervention that protects the victim in the room. If the therapist lets that question land, the session has become a place where the violence will be settled afterwards. **B9 Emeka:** (standing) I am not paying for this. **B10 Therapist:** You do not have to. (pause) Sit down if you want to hear what I am offering, or leave now and I will write to you. — The choice, without pursuit and without a threat. The therapist does not stand, does not raise her voice, and does not move towards the door. **B11 Emeka:** (after a pause, sitting) What are you offering. **B12 Therapist:** Individual sessions with me, or with a colleague if you prefer. A referral to a programme for men who want to change how they handle anger and control. And no conjoint sessions until that work has been done. — The offer is real, and the referral is to a programme with an evidence base rather than to "anger management", which is a different and largely ineffective thing. **B13 Emeka:** And her? **B14 Therapist:** She has the same offer. (pause) I want to be clear that I will not be passing messages between you. — The symmetry that is true and that protects the individual work, and the explicit refusal of the mediator role, which is the role a controlling partner will try to install.

What the transcript shows. The therapist never discloses, never accuses and never argues. She states decisions in the first person, protects the victim in the room with one sentence, and offers a real alternative. The one moment of real risk is B7, and the response to it is the shortest turn in the transcript.

9.7 Laboratory: the screening interview, observed

The exercise. Each student conducts a twelve-minute individual screening interview with an actor playing one of four briefed roles, and is then asked two minutes of questions by the observer. The roles are deliberately varied: a woman who minimises, a woman who is not being abused but whose partner is controlling about money, a man who is being abused by his wife and will not say so, and a woman who discloses strangulation in the last minute.

What is assessed. Whether the questions are asked at all; whether they are asked directly; whether the follow-ups are used; whether the lethality factors are covered;

whether the therapist's response to a disclosure increases or reduces the client's willingness to say more; and whether the safety planning begins.

Marks (20).

Criterion	0–1	2–3	4–5
Screening	Not asked	Asked in general terms	All four questions asked directly, with the strangulation follow-up
Response	Shock, advice, or "why did you not leave"	Neutral but passive	Names what was described, states it is not her fault, asks what she wants
Lethality	Not covered	Partly covered	Strangulation, threats, weapons, separation, escalation, children — all covered naturally
Safety	None	Advice given	A plan begun, with the "I am not telling you to leave" frame
Duty	Not mentioned	Mentioned vaguely	The child protection duty stated specifically, with what will be reported and when

The most common failure. The student stops after the first question because the answer is "no". The screening is not a checklist; it is a conversation in which the second and third questions do most of the work.

9.8 Six errors

Error 1 — Screening in front of the partner. The commonest and most dangerous error in the whole of couple work. The screening is individual, always, and the reason is stated so that it does not look like suspicion.

Error 2 — Accepting the couple's formulation. "We need communication help" is the referral, not the case. The formulation belongs to the assessment.

Error 3 — Continuing conjoint work after a disclosure of control. Sometimes because the victim begs for it, sometimes because the therapist believes she can manage the sessions. The sessions become the place where the violence is negotiated and, afterwards, enacted.

Error 4 — Telling her to leave. She has been told by her mother, her pastor and her husband. Her judgement is the only asset she has, and the therapist who overrides it has joined the list.

Error 5 — The report without the conversation. The call is made and the family learns of it from a social worker at the door. The relationship, and frequently the treatment, ends there.

Error 6 — The child interview conducted to establish the facts. Repeated questioning contaminates the evidence, harms the child, and does the investigator's job badly. One account, recorded verbatim, then the report.

9.9 Culture, faith and the household's rules about violence

The extended family's jurisdiction. In many of the households this book serves, a marital dispute is settled by elders, and a wife who takes it outside the family is breaking a rule that the family enforces. The therapist's question — "who in the family knows, and what did they say?" — is therefore not background; it is the risk assessment. A family council that has decided the matter is closed is a family council that will return the wife to the house.

Bride price and the language of ownership. Where a marriage involved a bride price, the language of the household may include the idea that the wife has been "paid for", and the husband's family may treat a separation as a debt. The therapist does not need to argue with the custom; she needs to know that the client may be hearing an obligation where the therapist hears a crime, and to make the distinction explicit: "the family may speak about what was paid. That does not change what is allowed to be done to you."

Religious teaching on submission. A client may hold a theology in which endurance is a virtue and divorce is a sin, and she may hold it sincerely. The therapist does not argue with the theology. She asks three questions: what does your faith say about what is being done to you; what does it say about your children watching; and who in your church would help if you told them? The answers frequently produce an internal permission the therapist could never have supplied.

The church as a resource and as a hazard. A congregation can provide practical help — a place to stay, money, childcare — and can also direct a woman back to a violent husband in the name of reconciliation. The therapist's task is to find out which, before recommending that the client tell her pastor.

The man's own shame. The perpetrator in many of these cases is not a stranger to shame; he is a man who has failed at work, at money, at status, and whose violence is the one arena in which he is not powerless. This is an explanation and not an excuse, and it matters clinically because a programme that addresses shame has a better chance than one that addresses anger.

The child's discipline. Physical discipline is normative in many households, and the boundary between discipline and abuse is set by the law and by injury, not by the therapist's cultural preference. The Child Rights Act and state laws define the threshold; the therapist applies it and does not negotiate it in the session, while recognising that the parent who is reported for a beating will experience the report as an assault on his parenthood.

> **DEBATE**
> "A therapist who tells a woman to leave a violent marriage is doing her job; one who does not is colluding." Argue for: safety is the first duty, the therapist has information the woman cannot have, and neutrality in the face of violence is complicity. Argue against: the woman's judgement about her own risk is more accurate than the therapist's, leaving is the period of greatest danger, and the decision must remain hers if the plan is to be used. Then write the sentence you would actually say, and the case in which you would depart from it.

9.10 Evidence note

What the research supports. The distinction between coercive control and situational violence is well established in the specialist literature and in practice guidance, and conjoint treatment is contraindicated where control is present. Strangulation is a strong predictor of subsequent homicide. Separation is a period of elevated risk. Specialist advocacy and safety planning improve outcomes for victims, and the evidence on perpetrator programmes is modest but consistent for court-mandated attendance. Screening increases identification; the evidence that screening alone improves outcomes is weaker, which is why screening must be paired with a response.

What is weaker. Prediction of lethality at the individual level remains limited. The evidence base on couple therapy in situational violence is small and contested. Child protection interventions have mixed evidence, with outcomes depending heavily on the quality of the service rather than the model.

What is not supported. That leaving resolves the risk — it often raises it in the short term. That a couple who present voluntarily are unlikely to have violence in the relationship. That a perpetrator's attendance at an anger management course changes behaviour. And that a child who is "too young to understand" is unaffected by exposure.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same disclosure of strangulation in a first session and in a twelfth changes the case entirely, because in the twelfth the therapist has already conducted six conjoint sessions.

9.11 Key terms

Coercive control — a pattern of domination through surveillance, isolation, threats and violence.

Situational couple violence — mutual, episodic aggression arising in conflict, without a pattern of control.

Lethality factors — the specific features associated with elevated homicide risk, including strangulation, threats to kill, weapons and separation.

Safety plan — an agreed set of preparations and contacts enabling a victim to act quickly if needed.

Advocacy — specialist practical support from a domestic violence service.

Threshold — the level of concern at which a report is required; reasonable suspicion of significant harm.

Exposure — a child's experience of violence between adults in the household, treated as harm in many frameworks.

9.11a The note that protects the next clinician

Five lines, written after every case in which violence or a child protection concern has appeared.

> Assessed individually on [date]. Client disclosed [strangulation on two occasions, most recent eight months ago; threats to kill; monitoring of phone and movements]. Lethality factors present: strangulation, threats, separation threat, escalation. Conjoint work contraindicated; couple informed [date], with reasons; individual work offered to both. Child protection referral made to [agency], [time], officer [name], advice received: [ ]. Safety plan agreed and recorded. Emergency contact protocol explained. Review [date]. Supervision [date].

The five lines exist for a reason that is not defensive: the next clinician, the social worker, the court and the therapist herself in six months all need to know what was known, when it was known, and what was done.

9.12 Self-test

1. Give four differences between situational couple violence and coercive control.
2. Why is conjoint work contraindicated in coercive control, and what are the three harms it produces?
3. State the four screening questions and three follow-ups.
4. Name six lethality factors and the one most strongly associated with homicide.
5. What is the "I am not going to tell you to leave" frame, and why does it work?
6. Give the eight parts of a safety plan.
7. What is the threshold for a child protection report, and what are the seven steps of the order of operations?
8. In the Okafor case, what did the therapist refuse to do, and why did each refusal matter?

Answers. (1) Pattern (mutual and episodic versus systematic domination); fear (situational versus chronic); mechanism (escalating arguments versus surveillance, isolation, money, threats); treatment (conjoint may be appropriate versus contraindicated). (2) Because it treats violence as a communication problem, gives the perpetrator access to the victim's disclosures, and produces retaliation after the session. (3) Hit, kick, push; made you afraid; forced sexual acts; safe going home today. Follow-ups: worst thing he has done; hands around your neck; threats to kill; escalation; monitoring. (4) Strangulation, threats to kill, access to a weapon, separation, stalking, escalation, pregnancy, sexual violence, history of violence outside the home, perpetrator instability; strangulation is the strongest non-fatal predictor. (5) An explicit refusal to direct the client, which preserves her judgement and makes the therapist usable; it works because she has been instructed by everyone else and the therapist's refusal distinguishes her. (6) Documents, money, bag, code word, route, children, phone, review. (7) Reasonable suspicion of significant harm. Order: immediate safety; record verbatim; consult; report; tell the parents unless that endangers the child; tell the child; continue treatment and record. (8) She refused to disclose Blessing's account, to accuse Emeka in the session, and to conduct conjoint sessions — protecting the victim from retaliation, preserving the confidentiality that made the disclosure possible, and removing the setting in which the violence would have been negotiated.

9.13 Reflective task

Write two hundred and fifty words on this question: a woman tells you she is not ready to leave and asks you not to raise it again. What do you say, and what do you record? Then answer honestly which of the two — saying it or recording it — you would find harder, and why. The difficulty is usually about helplessness, and helplessness is the feeling that produces either control or withdrawal in a therapist. Neither is the work.

9.14 Further reading

- Stark, E. on coercive control — the foundational account, and the source of the distinction this week rests on.
- Campbell, J. and colleagues on the danger assessment — the empirical basis for the lethality factors.
- The WHO clinical guidance on responding to intimate partner violence — practical and internationally adaptable.
- Any current national or state guidance on child protection reporting and the Child Rights Act, read alongside the state law that actually applies where you practise.
- A domestic violence advocacy service's own materials — read what the advocates give to women, because your client will read them too.

Next: Week 10 — The legal framework: courts, statutes and the therapist's place in them.

Week 10 — The legal framework a clinician must know

Learning outcomes. By the end of this week the student can (1) describe the four marriage systems operating in Nigeria and the dissolution route attached to each; (2) identify the five areas of law that bear on family therapy and state what the therapist may and may not do in each; (3) apply the welfare principle and explain why a treating therapist should not conduct a forensic custody evaluation of her own client; (4) route a legal question out of the session correctly, with a written referral question; and (5) explain why documentation is a legal instrument.

Orientation, not legal advice

This week teaches legal literacy, not law. Nothing here is advice, statutes vary between states and are amended, and the therapist who relies on a textbook rather than on a lawyer in a live case has made a category error. The purpose of the week is narrower and entirely practical: that the therapist can recognise a legal question when it appears in a session, knows which area of law it belongs to, knows what she may answer and what she must not, and knows who to send it to with a question that a lawyer can answer.

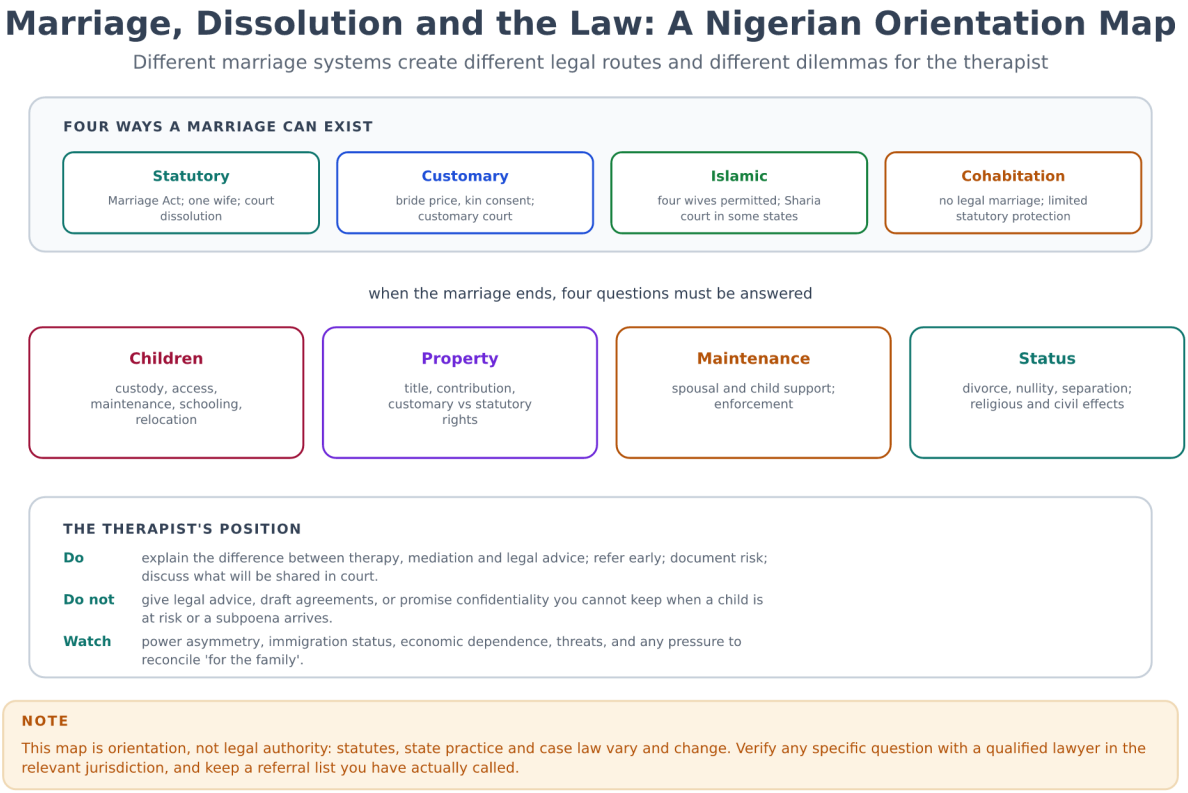


Figure 10.1 — Marriage systems and dissolution questions in Nigeria

Marriage in Nigeria exists in four overlapping forms: statutory marriage under the Marriage Act, customary marriage, Islamic marriage, and cohabitation without a legal marriage. A family may be married under more than one system at the same time, and the therapist who assumes a single legal framework will mis-hear the case.

System	What the therapist needs to know	What to refer
Statutory (Marriage Act)	Monogamous; dissolution by petition to the High Court under the Matrimonial Causes Act, with the two-year rule and recognised grounds	Grounds, timing, procedure, financial orders, custody
Customary	Varies by locality and by community; dissolution through customary process and customary courts; frequently no written record	Validity, dissolution, property consequences, the status of children
Islamic	Governed by Sharia law and Sharia courts in the states that operate them; dissolution by pronouncement, khul' or judicial process	Validity, divorce, maintenance (nafaqah), custody (hadanah)
Cohabitation	No legal marriage; limited statutory protection; the parties may believe they are married	Property, paternity, maintenance, inheritance

Why the plurality matters clinically, not only legally. A woman may believe she is married under the statute and discover that her husband's family has dissolved a customary marriage she did not know was the operative one. A man may marry a second

wife under Islamic law and be committing bigamy under the statute. A couple may have had three ceremonies and one legal marriage. The therapist's question is always the same: **which marriage, under which law, and what has been filed?**

10.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	Why the therapist needs legal literacy and must not give legal advice	Two cases: one where the therapist answered, one where she referred
8–25	The four systems and the dissolution routes	Diagram on the board; students place six families on it
25–40	Five areas of law and the boundary in each	Table, with a "may/may not" sort
40–55	The welfare principle; the custody report; the role conflict	Two reports, one by a treating therapist and one by an evaluator, compared
55–70	Routing a legal question: the five questions and the referral note	Pairs write a referral question from a case vignette
70–82	The Adewale case	Case, with students deciding what the therapist does
82–90	Documentation as evidence	One file note rewritten in legal-proof form

10.2 Five concepts that organise the week

Concept 1 — Legal literacy is a competence; legal advice is a different profession. The therapist must know enough to recognise, to route and to record. She does not opine on whether a marriage is valid, whether a divorce will be granted, whether custody will be awarded, or what a court will do. The distinction is not modesty; it is the boundary of the licence, and crossing it exposes the client to wrong advice and the therapist to liability.

Concept 2 — Legal pluralism means the family may be operating in two systems at once. Statutory, customary and Islamic frameworks coexist, and the same family may have obligations under more than one. The clinical consequence is that a client's account of "what the family has decided" may be legally real in one system and irrelevant in another. The therapist does not resolve this; she establishes which forum is operating and refers the question.

Concept 3 — The welfare principle governs children's cases, and it is not the same as the parents' wishes or the therapist's view of the parents. Every custody and maintenance decision is made by reference to the child's best interests. The therapist's contribution is evidence about the child, not advocacy for a parent. This is why the therapist's clinical opinion — "she is a good mother" — is not what the court needs; the court needs facts about the child's care, stability, schooling and relationships, and an assessment conducted for the purpose.

Concept 4 — Clinical and forensic roles are incompatible in the same case. A treating therapist cannot conduct a custody evaluation of her own client: the alliance compromises the neutrality the court requires, the client's disclosures are not gathered

for the purpose, and the therapist's opinion is shaped by months of one-sided narrative. The correct answer to the request is a factual letter and a referral to an independent evaluator (Week 6, section 6.7).

Concept 5 — Records are legal documents from the moment they are written. They can be subpoenaed, read aloud in court, quoted in a complaint, and inspected by a regulator. The note that records reasoning, dates, consent and actions is the therapist's evidence that she acted properly; the note that records impressions and speculation is the other side's evidence that she did not.

10.2a The five areas of law, with the boundary in each

1. Marriage and dissolution. The therapist may: ask which system or systems the marriage was contracted under, whether any petition or customary process has been started, and what the client wants. The therapist may not: advise on grounds, validity, timing or the likely outcome. Refer: to a family lawyer, with the question written down.

2. Children — custody, access, maintenance, guardianship. The therapist may: provide factual information with consent, support the child, and help the parents reach agreements that are not legally binding. The therapist may not: evaluate her own client's parenting for a court, recommend an outcome, or write a letter of support. Refer: to an independent forensic evaluator, and to a lawyer for maintenance and custody procedure.

3. Violence — criminal and protective. The Violence Against Persons (Prohibition) Act 2015 applies in the Federal Capital Territory and in the states that have adopted it; state laws vary in the protections they provide. The therapist may: explain her own reporting duties, support a client through a police statement, and refer to legal aid. The therapist may not: advise whether to report a crime, mediate a criminal matter, or promise an outcome.

4. Mental health legislation. The Mental Health Act 2021 reformed the framework nationally, and its adoption and implementation vary by state. It matters clinically because it governs involuntary assessment and treatment, and because a family may attempt to use it against a member. The therapist may: explain the process in general terms and support a client through an assessment. The therapist may not: initiate an involuntary process except through the statutory route and with legal advice.

5. Records and data protection. The Nigeria Data Protection Act 2023 and the professional codes impose duties about storage, access, retention and disclosure. The therapist may: hold records, share them with consent, and disclose where compelled or where a duty requires. The therapist may not: disclose without consent or legal authority, keep records indefinitely without a policy, or store clinical material in a form that can be read by a family member.

10.2b When litigation is pending: the therapy contract

The most demanding configuration in this week's material: the family is in court and the therapy is running at the same time. Four additions to the ordinary contract, agreed at the start and recorded.

1. The therapy is not part of the case. "This work is not evidence. If a court asks for my records, I will tell you and I will do what the law requires, and I will not be writing a report for either side." Stated at the start, it prevents the client recruiting the therapist as an ally, which is the commonest failure.

2. The court timetable is not the clinical timetable. The therapist asks for the dates of hearings and filings and writes them in the file. Sessions immediately before a hearing are different sessions — the client is mobilised, the children are frightened, and the material is often the material of the courtroom rather than of the family.

3. Nothing is said about the other party that cannot be said in front of them. This is a constraint on the therapist, not on the client: the therapist will not collude in building a case, and will name the collusion when it is invited ("you are asking me to agree that he is a bad father, and I am not going to do that").

4. The children are not witnesses. The therapist will not ask a child to report on a parent, will not carry messages, and will not tell a child what is happening in court beyond what the parents have agreed to tell them. Where the parents cannot agree, the therapist helps them compose a single short explanation, because the child who is told two stories is being asked to choose.

10.3 The plural system in the consulting room

The five configurations the therapist will meet.

1. **One marriage, one system.** The couple married under the statute and will divorce under it. The path is legible and the therapist's task is to keep the clinical work separate from the litigation timetable.
2. **One marriage, two systems.** A statutory marriage plus a customary ceremony, which many families treat as the "real" marriage and the registry as the formality. The dissolution question then becomes which forum the family will use, and the answer may differ between the husband's family and the wife's.
3. **Two marriages, sequentially.** A customary marriage followed by a statutory one, or the reverse. The legal position of the first wife may be entirely different from what she believes.
4. **Two marriages, concurrently.** An Islamic second marriage alongside a statutory first marriage, which may constitute bigamy under the statute — a criminal question, not a therapeutic one.
5. **No marriage.** Cohabitation, frequently described by the family as marriage, with children. Property, maintenance and inheritance questions arise, and the client may have no legal status at all.

What the therapist does with the configuration. Four questions, asked without legal opinion. Under which law was this marriage contracted, and is there a certificate? Has anything been filed or pronounced — a petition, a customary dissolution, a pronouncement? Who in the family has been involved in that process, and does the client agree with what they have done? What does the client want — the marriage, the divorce, the children, the property, or clarity?

The clinical significance of the fourth question. Clients frequently do not know what they want from the legal process, because the legal process has been started by somebody else — a husband's family, a brother, an employer. The therapist's work is to help the client separate the question "what is happening to me" from "what do I want", because a client who has not answered the second will be represented by whoever answers it for her.

The woman with no forum. In some of these cases the client has no effective access to any court: no money, no lawyer, no standing in the customary process, and a family that has already decided. The therapist's role then is practical and clinical rather than legal: identify the legal aid providers that exist, connect her with a women's organisation that accompanies clients, and support the psychological work of living with an outcome she did not choose. Doing this well is not a lesser form of help than referring to a lawyer.

10.3a The client who has been to three forums

A configuration that appears often enough to be its own subsection. The client has been to a customary court, to a pastor, and to a police station, and has three different answers. She arrives believing that "nobody will help", which is frequently accurate in the sense that no single forum has authority over the whole of her problem.

What the therapist does first. Builds the chronology, because the client's account will be organised by emotion rather than by date. A single page, with dates, forum, decision and who was present, converts a story of chaos into a set of events. The page is the client's, written with the therapist, and it is the most useful document the case will produce.

What the therapist does second. Establishes which forum is authoritative for which question. A customary court may have decided a matter of family status; a police report may have been closed; a pastor may have pronounced a reconciliation that has no legal effect. The therapist does not decide; she writes the question for each.

What the therapist does third. Names the pattern to the client. "You have been to three places and each of them answered a different question. That is why it feels as though nobody has answered you." Naming it is often the first moment at which the client feels understood, and it is the foundation for the referral that follows.

What not to do. Do not tell the client that her customary process "does not count". It counted, to the people who conducted it, and to her; the legal question about its effect is separate, and dismissing the process confirms that the therapist does not understand her world.

10.4 The welfare principle and the custody report

The principle. In any decision concerning a child, the child's best interests are the paramount consideration. The therapist does not need to know the statutory wording; she needs to know that the child's interests are not the same as either parent's wishes, and that a court will weigh stability, care, education, health, the child's own views where age permits, and the child's relationships.

What a court typically asks for, and what a therapist can provide.

The court wants	The treating therapist can supply	The treating therapist cannot supply
Facts about the child's care	Attendance, presenting problems, the child's own statements, with consent	An assessment of the other parent
The child's views	What the child has said, in context, if the child consents or the law requires	A recommendation about residence
Stability and functioning	Observations from sessions	A prediction of future parenting
Expert opinion	Nothing	An expert opinion — she is not the expert, and she is not independent

The factual letter, which is usually the right answer. On letterhead, with the client's written consent, it states: the dates of contact, the fact of attendance, the presenting problem in the client's words, and — only if the therapist judges it appropriate and the client agrees — a short statement of the clinical picture. It does not contain an opinion about parenting, an assessment of the other party, or a recommendation. It is dated, signed and sent to the court or the lawyer, and a copy goes in the file.

The request to appear. Where the therapist is subpoenaed she must attend and answer. Where she is asked to appear as an expert she must decline, and explain why: an expert opinion requires an assessment conducted for the court, independence from the parties, and a methodology that can be scrutinised. A treating therapist who offers an expert opinion in her own client's case is not being helpful; she is being replaced as a witness by an advocate.

The child's own voice. A child of twelve may express a preference, and the expression is evidence, not a decision. The therapist's task is to ensure the child is not placed in the position of choosing between parents, which is a burden that produces loyalty conflicts and lasting damage. The formulation that helps: "you do not have to choose. The adults will decide, and what you can tell me is what you want them to know about how it is for you."

The report that must never be written by a treating therapist. "In my opinion, the mother is the more capable parent and the children should reside with her." It is a forensic opinion, it is outside the therapist's role, it will be attacked in cross-examination, and it converts months of clinical work into a piece of advocacy that the other side can dismantle by showing the therapist met the father four times.

10.4a Routing a legal question: the five questions and the referral note

The moment it happens. A client says, "can he take the children from me?" or "if he has another wife, is our marriage not void?" or "my brother says I can divorce him for free in the customary court, is that true?" The therapist has two seconds in which her face will answer, and the answer must be a route rather than an opinion.

The five questions, in the session.

1. **"Has anything been filed?"** A petition, a customary process, a police report, a maintenance application. The answer changes the urgency from "sometime" to "within days".
2. **"Do you have a lawyer?"** If yes, the therapist's job is to help the client formulate the question for the lawyer. If no, the therapist's job is the referral, with the specific route.
3. **"What are you being asked to decide, and by when?"** A court date, a family meeting, a deadline for a response. Dates are the therapist's business even where the law is not.
4. **"Who else is a party?"** A husband's family, a co-wife, a child's guardian, an employer, an immigration authority. Multi-party cases need multi-party advice.
5. **"What do you want me to do?"** The client may want an opinion, a letter, a witness, or a person to sit with her. Each is a different request, and only some of them are ones the therapist can meet.

The referral note, which is the deliverable of the whole week. Six lines, written in the session, handed to the client, copied to the file.

> Re: [client], [date]. Thank you for seeing [client], who is considering/ facing [divorce petition / custody application / customary dissolution / maintenance claim]. She would like advice on: (1) the validity and effect of the [customary] dissolution pronounced on [date] in respect of a [statutory] marriage; (2) her position on residence of and access to the two children, aged 12 and 7; (3) whether maintenance can be sought and from which court. She is not able to fund private representation and has asked about legal aid. I am her therapist and am not able to advise on any of these matters. Yours, [name], [qualification], [contact].

Why the three numbered questions matter more than the letter's tone. A lawyer receiving "please advise on custody" will write a general letter that costs money and answers nothing. A lawyer receiving three numbered questions can answer them in fifteen minutes and will.

What goes in the file. The referral note, the date, the client's consent to the referral being made, and the question she asked in her own words. Nothing about the therapist's clinical view of the husband.

10.5 Building the referral network before you need it

Therapists build clinical networks and neglect legal ones, then improvise at the point of crisis. Five contacts, established in advance, each with a name and a number verified in the last year.

1. A family lawyer willing to take occasional pro bono or reduced-fee referrals, and willing to receive the six-line note of 10.4a rather than a telephone call in a panic.

2. A legal aid provider — the Legal Aid Council, a state scheme, a university law clinic, or a women's organisation with a legal desk. Legal aid is frequently available and almost never known about by the client.

3. The police family support unit at the nearest station, with the name of the officer in charge. This is the contact that determines whether a report is taken properly at eleven at night.

4. The state ministry responsible for social welfare, or the child protection unit, with the out-of-hours number if one exists (Week 9).

5. A mediator or a customary process you can respect. Not every case needs a court, and some families will use a mediation process that is cheaper, faster and more legitimate to them than litigation. The therapist should know which local processes are conducted fairly and which are instruments of the husband's family.

The maintenance of the network. One telephone call a year to each contact, and one case referred. A network that is only used is not a network; it is a list of strangers with whom the therapist is unfamiliar at the moment of greatest need.

10.6 Case: the Adewales, and the marriage that was also another marriage

The referral. Bola Adewale, 38, a schoolteacher, attends alone "for stress". She and Yinka, 42, a civil servant, have been married twelve years, with two children, 12 and 7. In session three she says, in passing, "his people have done what they wanted to do. They say we are no longer married."

What had happened. Bola and Yinka married in a church with a marriage certificate — a statutory marriage under the Marriage Act. In the eleventh year, following a dispute about money and Yinka's relationship with another woman, Yinka's family convened and pronounced a customary dissolution. Yinka moved out. He has continued to pay the school fees and has visited the children twice a month. He has told Bola that the marriage is over and that she should "go back to her family". Her brother has told her the same. She has no lawyer, has not been to any court, and believes that what the family pronounced is final.

The legal question, which is not the therapist's to answer. Whether a customary dissolution can dissolve a statutory marriage, and what Bola's position is in respect of the marriage, the children, maintenance and property. The therapist knows enough to know that this is a real question with a real answer, and does not know the answer.

What the therapist did.

1. **She asked the five questions.** Nothing filed; no lawyer; the deadline perceived by Bola was her family's pressure to move out "before the school term"; parties included Yinka's family and hers; and what she wanted was "to know whether I am still married".
2. **She wrote the six-line referral note** with three numbered questions, and telephoned a legal aid organisation she had a contact at, with Bola's consent, on the same day.
3. **She separated the three problems** that Bola had fused: the legal status of the marriage; the practical question of money and children; and the emotional question of whether her husband had ever intended to remain. Only the third was the therapist's, and it had been invisible because the first two were consuming her.

4. **She did not tell her to fight or to accept.** She said what she had said in other cases: "there is a legal question and I cannot answer it; there is a decision about what you want, and that is yours; and there is the grief, which is mine to work on with you."
5. **She recorded all of it,** including that she had made no legal assessment.

What happened. A lawyer at the legal aid organisation advised that a customary dissolution does not dissolve a statutory marriage, and that Bola remained married under the statute; a petition would be required. Bola did not immediately petition. She spent four months in treatment deciding whether she wanted to, and in the fifth month she filed. The therapist supported her through the process and did no legal work at all.

The clinical work, which is the part students underestimate. Bola's presenting "stress" was the experience of having been told by two families what her life was. She had not been asked what she wanted since she was twenty-six. The therapy's central question — "what do you want?" — was answered for the first time in session nine, in these words: "I want to be a married woman or a divorced woman. I do not want to be this."

What the case teaches.

1. **Legal pluralism produces a particular kind of helplessness,** and the helplessness is the clinical problem. The client who does not know whether she is married cannot plan, cannot grieve and cannot decide.
2. **The therapist's ignorance of the law is not a barrier when it is honest.** "I do not know, and here is how we will find out" is a complete clinical intervention.
3. **The family's pronouncement is not the law.** In this case the family's process was real to the family and had no effect on the statutory marriage. The therapist's refusal to accept the family's account as the legal fact changed the case, and she reached that refusal not by knowing the law but by knowing that a question existed.
4. **The three problems had to be separated** before any of them could be worked. This is the single most useful move in the whole week, and it generalises to almost every case in which a family presents with a legal crisis.

10.6a The factual letter, written out

The letter that is almost always the right answer to "will you write something for my case". On letterhead, with the client's written consent, five short paragraphs.

> **RE: [CLIENT NAME], [DATE OF BIRTH]; CONSENT TO DISCLOSURE DATED [DATE]**
>
> I have seen [client] on [n] occasions between [date] and [date], at her request, for [presenting problem in the client's words]. She has attended [n] of [n] scheduled appointments. The clinical picture is of [one or two sentences: anxiety following separation; grief; difficulty sleeping]. I have not assessed her parenting capacity and I make no recommendation about residence or contact. [Client] has told me that [only if relevant and consented: she wishes to see her children; she is attending a support group]. I make no finding as to the truth of what she has told me. If the court requires information about the children's welfare, I would suggest an independent assessment; I am not able to provide one, as I am [client]'s treating therapist and my view would not be independent. Yours faithfully, [name], [qualification], [registration number], [contact].

Why the third paragraph is written that way. "I make no finding as to the truth" is the sentence that keeps a clinical letter from becoming a witness statement. Clients often ask for it to be removed, because it weakens the letter; the therapist explains that its presence is what makes the rest of the letter admissible as an account rather than challengeable as advocacy.

The refusal to add the sentence the client wants. "In my opinion she is a fit mother." The refusal is delivered with the alternative: "I can say what I have observed, and I cannot say that. If you need an opinion, you need someone the court will treat as independent, and here is how to ask for one."

10.7 Transcript: "can he take the children?"

Session five, minutes eighteen to thirty-one. Bola has just been told that she remains married under the statute.

> **D1 Bola:** So can he take the children? His mother says they are his children because they bear his name. **D2 Therapist:** (pause) That is a legal question and I am not going to guess at it. (pause) I can tell you what I know and what I do not know. — The refusal, followed immediately by the structure of the answer. "I am not going to guess" is stronger than "I am not qualified", because it names what the therapist will not do rather than what she lacks. **D3 Bola:** But you must know. **D4 Therapist:** I know that there is a law about this, and that it is not the law his mother is describing. (pause) I do not know what a court would decide, and neither does she. — The two facts the therapist can honestly give: a law exists, and the family's version is not it. The second half is the clinically important one, because it interrupts the family's certainty without substituting the therapist's. **D5 Bola:** So I need a lawyer. **D6 Therapist:** You need a lawyer, and you needed one four months ago, and that is not your fault. (pause) I have a name, and I will telephone today if you want me to. — The acknowledgement that avoids blame, and the concrete offer. The client who is told "you should have got a lawyer" hears an accusation, and the accusation is almost always unjust. **D7 Bola:** I do not have money for a lawyer. **D8 Therapist:** There is legal aid, and I have checked that this organisation takes cases like yours. (pause) I am going to write the questions down for you, so that you are not paying for the first hour while he works out what you are asking. — The specific, prepared answer. Vague reassurance about legal aid is what most clients receive; a checked contact and three written questions is a service. **D9 Bola:** (long pause) If I am still married, then what has he been doing for a year? **D10 Therapist:** (pause) That is the question underneath the legal one, and it is the one we can work on. — The pivot to the clinical material, made explicit so that the client knows the therapist has heard it and has not dropped it. **D11 Bola:** He has been living as if I do not exist. And his family have been telling everyone in the compound that I am the one who refused. **D12 Therapist:** So you have been divorced in your street and married in the law. — The formulation that the rest of the case is built on, offered in one sentence and in the client's own terms. **D13 Bola:** (crying) Yes. (pause) That is exactly it. **D14 Therapist:** Then let us be clear about the three things we are dealing with: what the law says, which is your lawyer's; what you want, which is yours; and what it has been like to live in that gap, which is ours. — The separation of the three problems, stated aloud so that the client has a map she can use outside the room.

What the transcript shows. Seven turns, and in none of them does the therapist answer the legal question. In four of them she gives the client something usable: the existence of a law, the name of an organisation, three written questions, and a formulation of her situation that she recognises. The refusal is not the intervention; the substitution is.

10.8 Laboratory: legal triage and the written question

Part A — triage (twenty minutes, individual). Eight vignettes, each with a legal question embedded in clinical material. The student identifies: the area of law; whether the question is urgent (within seven days) or not; what the therapist may say; what the therapist must not say; and the referral destination.

Vignette	Embedded question
A wife says her husband's family "divorced" her by custom	Validity; effect on statutory marriage

Vignette	Embedded question
A father asks whether he must keep paying school fees after separation	Maintenance obligation
A mother says her husband has taken the children to his village	Custody; possible abduction
A client asks whether she can refuse her husband a second wife	Nothing legal; a marital and religious question
A couple ask whether their church marriage is "real"	Validity; registration
A client's employer demands a report about her	Data protection; consent
A client asks if he can be forced into psychiatric treatment	Mental health legislation
A client asks who will get the house	Property; matrimonial causes; customary law

Part B — the six-line note (twenty-five minutes, pairs). Each pair writes the referral note for one vignette, with three numbered questions, and reads it aloud. The group then marks it against four criteria: the questions are answerable; the note contains no legal opinion; it contains no clinical opinion that is not necessary; and it identifies the client's practical constraint (money, distance, language).

Marks (20).

Criterion	0–1	2–3	4–5
Identification	Wrong area of law	Right area, no urgency	Right area, urgency and forum identified
Boundary	Gave an opinion	Hedged opinion	Refused clearly and gave something usable instead
The note	A name only	A referral	Three answerable questions, practical constraints named
Record	None	Description	Question, consent, date, referral made, advice recorded

The comment to make in the debrief. The best notes in the room will be the shortest. Legal questions are answered by being narrowed.

10.9 Six errors

Error 1 — The reassuring opinion. "He cannot take the children; you are their mother." It is said kindly, it is frequently wrong, and the client will act on it. The error is not the warmth; it is the opinion.

Error 2 — Refusing to engage at all. "I am not a lawyer" said and nothing else offered leaves the client with the same problem and one fewer ally. The refusal must be followed by the route.

Error 3 — Treating the family's pronouncement as the legal fact. Customary processes are real to families and may have no legal effect. The therapist who accepts the family's version confirms the client's helplessness.

Error 4 — The letter of support. "I have known Mrs Adewale for a year and she is a devoted mother." It is not evidence, it will be cross-examined, and it has converted the therapist into a witness for one side.

Error 5 — The forensic evaluation conducted by the treating therapist. The role conflict described in 10.4, made worse by the therapist's belief that she knows the family better than an outside evaluator would.

Error 6 — The clinical note written for the file and read in court. Sarcasm, speculation, unverified allegation recorded as fact, and abbreviations that cannot be explained. The note is written for a reader who will be hostile. Write it that way.

10.9b The four questions a therapist asks her own lawyer

Therapists hesitate to consult a lawyer about their own practice because they assume it is expensive and that the question must be serious. In fact four questions account for most of the need, and each can be answered in a short consultation.

1. "May I disclose this, and to whom?" The records question. The answer depends on the statute, the consent and the order, and it changes with the jurisdiction.

2. "What must I do now that I have been served?" The subpoena question. The answer is almost always: verify, tell the client, obtain advice, produce only what is required, and keep a copy of everything produced.

3. "Am I obliged to report this?" The child protection and risk question, where the threshold is unclear.

4. "If I proceed as I intend, what is my exposure?" The advance question, asked before a decision rather than after it, and the one that most reduces the risk of a complaint.

The practice's arrangement. A retainer or a professional indemnity scheme that includes legal advice, and a named contact. The cost of the annual arrangement is a fraction of the cost of a single defended complaint, and the availability of advice changes the therapist's decisions in the direction of more honesty rather than less.

10.10 Culture, faith and the law the family actually uses

Customary process is not a lesser law; it is a different forum with different rules and real authority. In much of this region, a family's decision about a marriage is made in a room with elders, is binding in the eyes of everyone present, and is never written down. A therapist who treats this as "not real" will lose the family's confidence; a therapist who treats it as legally decisive will misinform the client. The correct position holds both: "what your family decided matters to them and to you. Whether it changes your legal position is a different question, and we will ask someone who knows."

The two forums and the confusion between them. Clients frequently assume that a customary dissolution ends a statutory marriage, that a church ceremony creates a legal marriage, that an Islamic marriage gives a second wife the same standing as the first, and that a father's family owns the children. Each assumption is common, each carries

consequences, and none of them can be corrected by the therapist's opinion. It is corrected by a lawyer's letter and by the client's decision to act on it.

Religion, obedience and the right to leave. A client may believe that her faith forbids divorce and that her suffering is a cross. The therapist does not argue with the belief. She does three things: she finds out what the client's own religious authority teaches about cruelty and about release, which is frequently more permissive than the client assumes; she separates the religious question from the legal one, since a religious prohibition is not a legal incapacity; and she asks the question that opens the case: "if your faith permitted it, what would you want?"

The family's use of the law as a threat. "My husband says he will take the children and I will never see them." Sometimes the threat is knowledge of the law; more often it is a threat with no legal basis, issued to produce compliance. The therapist cannot assess which it is. She can do two things: name the threat as a threat, which reduces its power, and route the client to advice that will tell her the truth.

The honour dimension of a public case. Where the family's standing depends on the marriage being intact, a court case is a public declaration that it is not. Clients may therefore prefer a private, customary or mediated route even where a court would give them more. That preference is rational and the therapist should respect it, while making sure the client knows what she is giving up.

The male client and the law's reputational cost. A father who is seen to be fighting his wife in court may lose standing at work and in his family. The therapist working with such a father should not read his reluctance to litigate as indifference to his children.

> **DEBATE**
> "A family therapist should always advise a client to obtain legal advice before making any decision about the marriage." Argue for: information is a precondition of consent, the stakes are irreversible, and the therapist's silence can be read as endorsement of the family's version. Argue against: the client may not want legal advice, the referral may itself escalate the conflict, and the therapist's role is to support the client's own decision rather than to steer it. Then state the three cases in which you would insist.

10.11 Evidence note

What is known. Legal pluralism is a structural feature of family life in Nigeria and in much of sub-Saharan Africa, and the literature on family law in plural systems documents precisely the confusions described in this week: conflicting adjudications, women's differential access to forums, and the gap between customary practice and statutory protection. Research on help-seeking in domestic violence consistently finds that legal literacy is one of the strongest predictors of a woman taking protective action, and that the absence of accessible advice is a principal barrier. Studies of client experiences of court-involved therapy find that role confusion — treating clinician versus evaluator — is a common source of complaint and of harm.

What is weaker. There is little empirical work on the specific question of how therapists should handle legal plurality in the consulting room, and almost none on outcomes. The guidance in this week is therefore derived from professional ethics, from the structure of the law, and from practice rather than from trial evidence.

What is not supported. That a therapist's clinical opinion about parenting has predictive validity for custody outcomes. That legal advice from a non-lawyer is harmless if it is well-intentioned. And that referral to a lawyer inevitably escalates a family's conflict — in many cases it reduces it, because each party stops guessing.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Legal advice given in week two, before positions have hardened, and in week twenty, after a customary dissolution has been pronounced and acted upon, produce two different families.

10.11a The six documents a family therapist should hold

Legal literacy is partly a matter of paperwork, and the paperwork is cheap.

1. **A consent form** that names the limits of confidentiality, the duty to report, the possibility of records being compelled, and the identity of the client system — reviewed and re-signed when the system changes (Week 2).
2. **A privacy and records policy** stating what is kept, where, for how long, who may see it, and how a request for access is handled.
3. **A disclosure protocol** for requests from lawyers, courts, employers, insurers and the police: who decides, what is checked, what is recorded.
4. **A subpoena procedure,** one page: verify the document, tell the client, contact the supervisor or indemnifier, seek legal advice, produce only what is required.
5. **A complaints procedure** telling the client how to complain about the therapist, because a client who has a route to complain is a client less likely to litigate.
6. **A list of verified contacts** — lawyer, legal aid, police family unit, social welfare, crisis line — with the date each was last checked.

The file that survives scrutiny is not the file with the most detail. It is the file in which every disclosure has a consent behind it, every referral has a question attached, and every decision has a date.

10.12 Key terms

Legal pluralism — the coexistence of statutory, customary and religious legal systems within one jurisdiction.

Matrimonial Causes Act — the statute governing statutory divorce in Nigeria, including the two-year rule and the recognised grounds.

Welfare principle — the rule that the child's best interests are the paramount consideration in decisions concerning children.

Forensic role — the role of an independent expert providing an opinion to a court.

VAPP Act 2015 — the Violence Against Persons (Prohibition) Act, applicable in the FCT and in adopting states.

Mental Health Act 2021 — the national framework for mental health care, with state-level variation in implementation.

Legal aid — publicly funded legal advice and representation; availability varies and is frequently unknown to clients.

10.13 Self-test

1. Name the four marriage systems and the dissolution route for each.
2. Give the boundary, for the therapist, in each of the five areas of law.
3. What is the welfare principle, and what does it require of a therapist asked for a report?
4. Why should a treating therapist not conduct a forensic custody evaluation of her own client?
5. State the five questions that route a legal question.
6. What is the six-line note, and why three numbered questions?
7. In the Adewale case, what were the three problems, and which was the therapist's?
8. Why is the family's pronouncement not the law, and why does that matter clinically?

Answers. (1) Statutory — petition to the High Court under the Matrimonial Causes Act; customary — customary process and customary courts, locality-specific; Islamic — pronouncement, khul' or Sharia court; cohabitation — no marriage, so limited statutory protection. (2) Marriage — may ask which system and what has been filed, may not advise on validity or grounds; children — may give factual information with consent, may not evaluate or recommend; violence — may explain her own duties and refer, may not advise on reporting or mediate a crime; mental health — may explain and support, may not initiate involuntary processes outside the statute; records — may hold and share with consent, may not disclose without authority or keep records indefinitely without policy. (3) The child's best interests are paramount; it requires the therapist to give facts about the child and to refuse an opinion about parenting or residence. (4) Because the alliance and the one-sided narrative compromise independence, the material was not gathered for the purpose, and the opinion will be dismantled in cross-examination. (5) Has anything been filed; do you have a lawyer; what must be decided and by when; who else is a party; what do you want me to do. (6) A short referral letter with three numbered, answerable questions; because a general request produces general advice that costs the client money and answers nothing. (7) The legal status of the marriage; the practical questions of money and children; and the emotional question of whether her husband had ever intended to stay — only the third was the therapist's. (8) Because a customary process may have no legal effect on a statutory marriage, and clinically because the client's belief that her life has been decided by others is the presenting problem; correcting it restores her capacity to decide.

10.9a The therapist in the witness box

Most family therapists will give evidence at some point, usually in a custody or protection matter, and the experience is unlike anything else in the work.

The four rules. Answer the question asked. Not the question behind it, not the one you would prefer, and not the narrative you have prepared. Say "I do not know" when you do not know, which is more credible than an improvisation and is frequently the honest answer. Do not defend your clinical decisions beyond the point of explanation. A therapist who argues with counsel is winning an argument and losing the case. Take your notes, and refer to them, because a witness who says "I would need to check my notes" and then reads them accurately is more reliable than one who reconstructs from memory.

The three questions that will be asked. "You only ever saw one side of this family, did you not?" "You are not a psychiatrist, are you?" "You formed the view that my client was the problem because that is what you were told." Each is answerable honestly and none requires a defence. The honest answers — "yes, I saw both parents but mostly the mother", "no, and I have not offered a psychiatric opinion", "I formed my view from what I observed and I recorded what I was told" — are stronger than any attempt to sound more authoritative than the therapist is.

The preparation. Read the file in full; write a chronology; identify the limits of your knowledge; discuss with your supervisor what you will say about the decisions you made; and remember that the court is not asking whether you are a good therapist. It is asking what you know.

10.14 Reflective task

Take one of your own family cases from placement, or the Adewale case, and write three hundred words separating the clinical question from the legal one. Then answer: which of the two did you spend more time thinking about, and why? Most trainees find that the legal question is more anxiety-provoking because it has a right answer and they do not know it — which is exactly the anxiety that produces either an opinion or a refusal, and neither is the work.

10.15 Further reading

- The Marriage Act, the Matrimonial Causes Act and the Child Rights Act, read in the original and not in summary — they are shorter than students expect.
- The Violence Against Persons (Prohibition) Act 2015, with attention to the states that have adopted it.
- The Mental Health Act 2021 and the state-level implementation guidance where you practise.
- Any current text on family law in Nigeria, for the reference section of the practice manual.
- The Nigeria Data Protection Act 2023 and your professional body's record-keeping guidance.

Next: Week 11 — Records, subpoenas and the court.

Week 11 — Records, subpoenas and court

Learning outcomes. By the end of this week the student can (1) write a record that serves three readers — her future self, a colleague taking over, and a court; (2) distinguish fact, observation, clinical opinion and hypothesis in a note; (3) handle an access request from a client, a partner, an employer or a court; (4) respond to a subpoena in the correct order and with the correct advice; and (5) give evidence as a witness of fact without becoming an advocate.

The record is two things at once

It is the clinical memory of the treatment, and it is a legal document that may be read by people who have no interest in the therapist's good intentions. The two functions are compatible, and the note that serves the first serves the second: a note that records what was observed, decided and done is both the best clinical note and the best evidence. Notes fail when a therapist writes for one reader only — the impressionistic note written for herself, or the defensive note written for a regulator — and the failure shows in both places.

Write for three readers.

Reader	What they need
The therapist in six months	What was the problem, what was tried, what happened, what is planned
A colleague taking over tonight	The risk, the current plan, who knows, what to do if telephoned
A court	Facts with dates, sources, decisions with reasons, consent, and no speculation

The four registers, which must never be mixed in a single sentence.

1. **Fact.** "Client attended on 4 March. Her left forearm was bandaged."
2. **Observation.** "She was tearful at the start of the session and calm by the end. She checked her phone four times."
3. **Clinical opinion.** "In my view the presentation is consistent with an acute stress response following discovery of the affair."
4. **Hypothesis.** "Possible that the husband's drinking is a factor; not yet explored."

A note that says "client is a manipulative woman who is clearly exaggerating her husband's behaviour" contains no register at all: it is a character judgement, it is unsupported, and it will be read aloud in a witness box.

11.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The three readers; two notes on the same session, compared	Read both, vote on which is defensible

Minutes	Content	Teaching form
8–22	Five concepts: memory, hostile reader, four registers, minimum necessary, privilege	Lecture
22–40	Note structure, with a worked example written live	Therapist writes a note on the board from a vignette
40–52	The multiperson record: whose file, whose secret, where it goes	Three configurations, students place the material
52–65	Access requests and the subpoena sequence	Simulation: a letter arrives, what do you do in the next hour
65–80	Giving evidence: the four rules, the three questions	Mock cross-examination with a colleague as counsel
80–90	The Eze subpoena	Case, students decide what is produced and what is withheld

11.2 Five concepts that organise the week

Concept 1 — The record is the treatment's memory, and memory is a clinical instrument. Continuity of care depends on the next clinician being able to reconstruct what happened. A file of one-line entries ("seen; discussed anxiety; better") is not a record of a treatment; it is a record of attendance, and it cannot support a handover, a supervision, or a defence.

Concept 2 — Assume a hostile reader. Not because the reader is hostile, but because the only readers who matter in a dispute are the ones looking for inconsistency. Write as though the sentence will be read aloud by someone who wants it to mean something else, because one day it will be.

Concept 3 — The four registers must be labelled. Facts stated plainly, observations stated as observations, opinions introduced as opinions, hypotheses marked as hypotheses. The failure to distinguish them is the single commonest reason a clinical record damages the clinician who wrote it.

Concept 4 — Minimum necessary is the rule for every disclosure. To a court, to a school, to a physician, to a partner: disclose what is required for the purpose and nothing else, and record what was disclosed and why. A file sent in full to a school because the school asked for "the records" is a breach, whatever the school's intention.

Concept 5 — Privilege and confidentiality are different, and privilege is not the therapist's. Confidentiality is the professional duty not to disclose. Privilege is the client's legal protection against disclosure in proceedings. In most jurisdictions privilege is qualified: it can be waived by the client, overridden by statute, or displaced where the court orders production. The therapist cannot promise privilege, which is why the consent conversation must mention that records can be compelled.

11.2a The note, written out

The structure for a family session. Six headings, five to eight minutes of writing, and the note is done.

> **Attendees:** Mrs Eze, Mr Eze (first 20 minutes only), therapist. Children not present. **Presenting this session:** Mr Eze raised the school's report of Chidi's absences; Mrs Eze stated she had not been told of the meeting. **Risk screen:** No disclosures of harm this session. Direct enquiry about safety at home — client states no violence in the last month; no suicidal ideation on direct enquiry. **Interventions:** Reviewed the previous week's agreement about school communication; both parents agreed a written message each Friday; therapist modelled the message and both rehearsed it. **Response:** Mr Eze left the session at minute 20 saying he had to return to work. Mrs Eze remained and stated that she had stopped telling him about school matters "because it always ends in a quarrel". **Plan:** Weekly sessions continue. Mrs Eze to attend alone next week, as agreed in the room. School's safeguarding lead to be told only that the family is attending; consent for that limited contact recorded.

The same session, badly written. "Good session. Couple engaged. Father defensive and clearly controlling; mother tearful and possibly depressed. Will continue." Four judgements, no facts, no plan, no consent, and each judgement is an assertion the therapist cannot support from the session.

The six things every note should contain. Date and attendees; the reason for the session; the risk screen, with the questions asked; what the therapist did; what the client(s) did in response; and the plan, with who was told what.

The eight things that must never appear.

1. **Character judgements** — "manipulative", "attention-seeking", "a difficult woman".
2. **Speculation about third parties** — "I suspect the grandmother is behind this".
3. **Unverified allegation recorded as fact** — "mother is an alcoholic" when the source is the husband.
4. **Other clients' information.**
5. **Secrets** — an individual disclosure recorded in a file to which the other partner may have access.
6. **Jokes, irony, nicknames,** and anything the therapist would not want read aloud.
7. **Abbreviations that cannot be expanded** by a reader who is not the author.
8. **Opinions about the law or the likely outcome of a case.**

The note written late. It happens, and the honest entry is "note written on [date] from memory of session on [date]". An undated, unmarked reconstruction presented as contemporaneous is the error, not the lateness.

11.2b The multiperson record: whose file is it?

The question that generates more complaints than any other in couple and family work.

The three configurations.

Configuration	The file	The individual disclosures
Individual client, family as context	The client's file	In the file; the family has no access
Couple as client, one file	One file, both named	Individual disclosures recorded in a **restricted section**, accessible only to the therapist and supervisor
Family as client, children included	One file with the children named as clients	Children's disclosures recorded separately, with a note of who may see them

The restricted section: how it works in practice. A separate, clearly marked section of the record — in an electronic system, a separate note type with restricted access — containing material given in individual sessions that the other party has not been told. The therapist's clinical record is complete; the disclosure risk is contained. Where the practice has no electronic system, the equivalent is a sealed envelope referenced in the main file, with the practice policy stating what it contains and who may open it.

Why this matters more than it appears. In a couple case the partners may each have a right of access to the data held about them under data protection law, and the other partner's disclosures are data about them in part. The therapist who has recorded the wife's account of the husband in the joint file, in a form that identifies him, will find herself in the position of having to disclose it or to justify withholding it. The restricted section and the practice's policy are what make that possible.

What the note should say when restricting. "Individual session [date]: material recorded in restricted section per practice policy [reference]; client informed that this material will not be shared with [partner]; no information affecting safety withheld from the file." Three clauses, and they answer the question a regulator will ask.

11.2c The electronic record

Most practices now keep records electronically, and the electronic record creates five problems that the paper record did not.

1. The audit trail remembers everything. Every version, every deletion, every time a note was opened and by whom. A therapist who "tidied" a note six months after a session will be asked why, and "for clarity" is not an answer that survives the question.

2. The shared device. A practice laptop used at home, a phone with the notes app, an email with clinical content in it — each is a disclosure waiting to happen. The policy is simple: clinical records are kept on the clinical system, nothing clinical is in personal email, and no clinical material is on a personal phone.

3. The copy that leaves the building. A client who is sent a summary by email has a copy that the therapist cannot retrieve. Where a summary is sent, it is written for that purpose, marked as such, and recorded.

4. The template that writes the note. Templates improve completeness and produce homogeneity: every note says "client engaged well" because that is the first option in the

dropdown. The discipline is to use the template's headings and the therapist's own sentences.

5. The access request that is a download. Where a client requests their data, the practice must be able to produce a coherent record rather than a raw export that includes other clients' names. The restricted section of 11.2b is, in an electronic system, an access permission — and the permission must be tested before it is needed.

11.3 Access requests: four kinds, four answers

1. The client's own request. Under data protection law the client is generally entitled to a copy of the data held about them, subject to exceptions. The therapist's task: verify identity, establish whose data is involved, redact third-party information, and respond within the statutory period. Where the therapist believes access would cause serious harm, the statutory exception may apply and legal advice should be taken before refusing.

2. The partner's request in a couple case. The partner is also a client in a jointly-held file, which is why the restricted section exists. The therapist discloses what is about that partner and does not disclose the other partner's individual material. The reasoning is recorded.

3. The employer's, insurer's or school's request. Usually limited to attendance, or to a report with written consent. The therapist discloses the minimum and records it. An employer who asks for "the notes" receives attendance only, and is told why.

4. The court's or the police's demand. Weeks 10 and 11's core business, and the subject of 11.4.

The letter that answers an access request. Five lines: what has been received, what will be provided, what will be withheld and on what basis, when it will be sent, and who to contact. The therapist does not argue in the letter; she states and records.

The request to "just tell me how she is doing". The commonest of all, made by a relative at a wedding. The answer is one sentence, delivered warmly: "I am not able to talk about that, and I know it is because you are worried about her."

11.3b The client who asks to see her file

The request arrives in three forms, and each is answered differently.

"What have you written about me?" The most straightforward, and the one that is frequently met with evasion. The answer: "I will show you." Reading the record together — the notes, the plan, the supervision entries — takes twenty minutes and does three things: it demonstrates that the therapist has been paying attention, it allows the client to correct factual errors, and it removes the sense that something is being written behind her back.

"I want a copy." A data protection request, answered within the statutory period, with third-party information redacted and a short covering letter. The therapist does not require a reason, does not negotiate, and does not express hurt. The one legitimate

caution is where access would cause serious harm, which is a legal exception to be taken on advice and not a feeling to be acted on.

"My husband wants to see what you wrote about him." If he is a client in the same file, he may be entitled to what is about him. What is about his wife is not his, and the restricted section exists for that reason. The therapist replies in writing, stating what will be provided and what will not, with the basis.

The clinical value that is usually missed. Reading the file with a client is one of the few interventions that reliably produces a correction of the therapist's misunderstanding. Clients routinely say, at line three, "that is not what I meant" — and the correction is worth more than the twenty minutes.

11.4 The subpoena: what it is and what to do in the first hour

What it is. A court order requiring the production of documents or the attendance of a witness. It is not a request, it is not optional, and ignoring it is contempt. It is also not, in itself, an order that the therapist disclose everything in the file: it defines what is required, and the therapist's obligation is to comply with what it requires and nothing beyond.

The five checks, in the first hour.

1. **Is it valid and addressed to me?** Names, addresses, the court, the case number, the date, and the signature or seal. A photocopied letter from a lawyer is a request, not an order, and the two are answered differently.
2. **What exactly does it require?** Documents, attendance, or both; the date; and the scope — "all records relating to" is broader than "your notes of 4 March" and both must be read precisely.
3. **Has the client been told?** Unless the order prohibits it, the client is informed immediately, in writing, and the therapist explains what will be produced.
4. **Who is advising me?** The supervisor, the indemnity provider, the professional body and, where necessary, the therapist's own lawyer. The therapist does not respond alone to a legal process.
5. **What is the deadline, and is there time to object?** Where the scope is excessive or the material privileged, the advice may be to seek to limit the order. That application is made by a lawyer and it is made before the deadline, not after.

The order of operations once advice is taken.

- Produce only what is required, in the form required.
- Keep a complete copy of everything produced, indexed, and mark the date it was produced.
- Where something is withheld on advice, say so in writing and on what basis — a short privilege log.
- Record the process in the file: what was received, when, what advice was taken from whom, what was produced, when, and to whom.

- Tell the client what was produced, unless prohibited, and record that you did.

What not to do. Destroy or alter records, which is a serious offence and the end of the therapist's credibility. Produce the entire file because it is simpler. Ring the other side's lawyer to negotiate scope without advice. Or promise the client that nothing will be produced.

The extra care in family cases. The file frequently contains the other party's individual material and sometimes a child's. The therapist produces what the order requires about the client she treated, and where the order is broad enough to capture other people's disclosures, she raises the scope with her adviser. This is not obstruction; it is the discharge of duties owed to people who are not parties to the dispute.

11.4a Privilege, and the promise a therapist must not make

The distinction. Confidentiality is a duty the therapist owes. Privilege is a right the client holds, protecting communications from disclosure in legal proceedings. The therapist's obligation is to keep confidence; the client's privilege may be waived by the client and may be overridden by statute or by the court.

The four situations in which privilege will not save the file.

1. **Where the client waives it,** frequently by putting their mental state in issue — a client who claims psychological injury, or who relies on the therapy as evidence of good parenting, has opened the door.
2. **Where a statute creates an exception,** child protection being the commonest.
3. **Where the court orders production** after considering any claim of privilege.
4. **Where the therapist is the subject of a complaint or proceeding,** in which case the record is the evidence the therapist herself needs. The codes generally permit disclosure for the purpose of defending a complaint.

The consent conversation, revised. The Week 2 sentence — "this is confidential except for the limits I am about to describe" — should include one further clause in any case with a legal dimension: "and if a court ever orders me to produce my records, I will tell you, I will seek advice, and I will produce what the order requires."

What the therapist must never say. "Nothing you tell me can ever be used in court." It is false, it is frequently said to reassure, and it is the false promise on which a client later bases an allegation of betrayal.

11.4b The request from the school

The commonest third-party request in family work, and the one most often answered carelessly.

What the school actually needs, and what it does not. The school needs to know that the child is receiving support, whether there are practical implications for school life, and — where the school is a referrer — whether to continue the referral. It does not need the content of the therapy, the parents' accounts of each other, or a diagnosis the therapist is not qualified to give.

The three permitted answers. Nothing, where the family does not consent, except where a safeguarding duty requires otherwise. Attendance only, where the family consents to that and nothing more: "the family is attending and I have their consent to tell you that." A short agreed summary, written with the family, showing them the text before it is sent.

The safeguarding exception, handled carefully. Where the school's involvement is a child protection matter, the therapist may be required to share information with the safeguarding lead. That sharing is specific, recorded, and — unless telling the parents would endanger the child — done with the parents' knowledge.

The request that arrives by email at 4 p.m. on a Friday. The answer is not a reply. It is an acknowledgement that the request has been received, that consent will be checked on Monday, and that the therapist will respond in writing. Most breaches in small practices happen in the twenty minutes before a weekend.

11.5 Giving evidence

The role. A witness of fact reports what she saw, heard and did. She does not advocate, does not offer opinions outside her expertise, and does not answer the question she would have preferred. Where the therapist has also been appointed as an expert — rare, and never in her own client's case (Week 10) — the rules are different and the distinction must be stated at the outset.

The four rules, in the box.

1. **Answer the question that was asked.** Briefly, then stop. The commonest error is the therapist who, anxious to be understood, continues into material that creates a new line of attack.
2. **Do not guess.** "I do not know" and "I would need to check my notes" are complete and creditable answers. An improvisation under pressure is how witnesses are destroyed.
3. **Distinguish fact from opinion, and say which you are giving.** "I observed X. My opinion, for what it is worth, is Y, and I should say I am not a psychiatrist."
4. **Do not defend the treatment.** Counsel will invite the therapist to justify a decision. The answer is an explanation, not a defence: "I made that decision because at the time I had these three pieces of information, and I recorded it here."

The three questions that will come.

"You only ever saw one side, didn't you?" Answer honestly: how many sessions each party attended, and what the therapist did not have.

"You are not a doctor, are you?" Answer honestly, and state what the therapist is qualified to say — which is usually about the therapy, not the diagnosis.

"That is your opinion, and it is only an opinion." Agree, and say what it rests on: the sessions, the notes, the recorded statements, the supervision. The opinion that is supported by evidence survives this question; the opinion that is a feeling does not.

The preparation that matters. Read the whole file, not only the notes about the session in question. Write a chronology. Mark the three decisions you might be challenged on and be able to explain each in three sentences. And rehearse with a colleague playing counsel, because the first time the question is asked should not be in court.

The day itself. Take the file and the order. Arrive early. Dress as the court expects. Speak to the bench, not to counsel. Answer slowly. Ask for a break if you need one. And remember that the discomfort of the witness box is not evidence that the therapist did something wrong.

11.5a The report written before the hearing

Where the court asks for a report, the structure below produces something usable and honest. It is a report of fact and clinical opinion within the therapist's competence, not a custody evaluation (Week 10).

The six sections. The instruction: who asked for the report, what was asked, and the date. The sources: sessions, dates, who attended, documents seen, and what was not available. The history: brief, factual, and drawn from the record rather than from memory. The observations: what the therapist saw, in behavioural language. The clinical opinion: clearly labelled, bounded by the therapist's competence, and stating its limits. The limits: what the therapist cannot say — parenting capacity, the other parent, prognosis, the child's best interests.

The four sentences that keep a report inside its role. "I have not assessed [the other parent]." "I make no recommendation as to residence or contact." "This report is based on my contact with [client] over [n] sessions and on no other source." "I am willing to attend court and to answer questions about the treatment I provided."

What the report must not contain. A recommendation disguised as an observation ("she was consistently the more attentive parent, which I would expect to be relevant to residence"), the other party's alleged behaviour presented as fact, and any material from an individual session of a partner who has not consented.

11.6 Case: the subpoena in the Eze matter

Background. Mrs Ngozi Eze, 38, was in treatment for eight months following her husband's departure (Week 6, section 6.7, where she asked the therapist to be a character witness and was refused). She is now in a custody dispute over two children, 11 and 6.

The document. A subpoena duces tecum arrives by courier, addressed to the therapist, requiring "all notes, records, correspondence and reports relating to the treatment of Ngozi Eze" and her attendance on a date three weeks away. It is issued at the request of the husband's solicitor.

The five checks. Valid and properly addressed; scope extremely broad; client not yet told; supervisor available, indemnity provider contacted; deadline leaves eleven days to seek advice on scope.

What is in the file, and why it matters.

1. The treatment notes across eight months, including Mrs Eze's account of the marriage, which is highly critical of her husband and includes allegations that have never been tested.
2. Three entries in which she describes her mother-in-law supervising the children and says she "does not trust her".
3. A note recording that Mrs Eze told the therapist, in an individual session, that she had once left the children with a neighbour overnight while she travelled to another city for a funeral. The therapist recorded it as a fact reported, not investigated.
4. A letter the therapist wrote at Mrs Eze's request to the children's school, confirming that the family was attending therapy, with no clinical detail.
5. A supervision note recording the therapist's view that Mrs Eze was "fragile but improving".
6. Nothing about the husband, who was never a client.

The advice received. Produce items 1, 3, 4 and 5 in response to the order, with third-party identifiers redacted where they are not the subject of the proceedings (the neighbour's name, the mother-in-law's medical details, which appear in one sentence about her own health). Item 2 is producible, with the mother-in-law's health information redacted. Item 5's reference to "fragile" is producible and the therapist should expect to be asked about it. Nothing is withheld except the redactions, and the redactions are logged with reasons.

What the therapist did.

1. Told Mrs Eze by telephone the same day, in the words: "your husband's lawyer has asked the court to order me to produce my records. I have to comply. I am going to tell you exactly what is in the file and what will be produced, and I am going to take advice first." She then read the relevant entries to her, over two sessions.
2. Wrote to Mrs Eze confirming the conversation, the scope of the order, what would be produced, and the date.
3. Took advice from her indemnity provider and, through them, a lawyer, on the redactions and on whether to seek to limit the scope. The advice was to produce with redactions and not to contest the scope, because the material was plainly relevant and a contested application would delay the hearing and damage the therapist's position.
4. Produced an indexed bundle, paginated, with a covering letter stating what was produced, what was redacted, and the basis for each redaction.
5. Attended and gave evidence.
6. Recorded all six steps in the file, with dates and names.

The evidence, and the two hard moments.

The first. Counsel read out the note about the overnight stay with the neighbour. "You recorded that my client's wife left the children overnight with a neighbour, did you not?" The therapist answered: "I recorded what she told me. I did not investigate it and I made no finding." Counsel: "So you cannot say whether it is true." Therapist: "That is correct."

The second. Counsel read out the supervision note: "fragile but improving". "You considered her fragile." Therapist: "I wrote that in a supervision note eight months ago, in the context of her grief and her financial position. I also wrote 'improving', and the treatment continued for five months after that note. I can explain the context if the court wishes."

The outcome of the hearing, which is not the point of the case. Residence was ordered with the mother, with defined contact for the father. The therapist's evidence neither rescued nor destroyed the case, which is what evidence from a treating clinician usually does.

What the case teaches.

1. **The order was complied with, not resisted, and the compliance was careful.** The therapist's protection was not argument but accuracy: an indexed bundle, logged redactions, and a recorded process.
2. **The client was told before she was surprised.** The two sessions spent reading the file to her were the clinical work of the week, and probably prevented the treatment from ending.
3. **The notes she had written were the evidence.** The "fragile but improving" note was the only moment of real difficulty, and it was survivable because it was a supervision note with a date and a context, not a character judgement in the clinical record.
4. **The file contained material about people who were not parties,** which is the ordinary condition of family work and the reason the redaction log exists.
5. **Nothing was destroyed, altered or withheld without a reason on the record.** Had any of those three happened, the case would have been about the therapist rather than about the children.

11.7 Transcript: fifteen minutes of cross-examination

Condensed from the hearing, with annotation.

> **X1 Counsel:** You are not a psychiatrist, are you? **X2 Therapist:** No. I am a family therapist, registered with [body]. **X3 Counsel:** And you are not qualified to say whether my client's wife is a fit mother. **X4 Therapist:** I have not said that, in my notes or in this court. — Fourteen words. The answer corrects the premise without arguing with counsel, which is the difference between a witness and a combatant. **X5 Counsel:** You formed a view, though. **X6 Therapist:** I formed a clinical impression of a woman in grief, which is what I was treating. (pause) I recorded it and I have explained what it was based on. — The concession (yes, a view) followed by the limitation (of a particular thing), which is more credible than a denial. **X7 Counsel:** You never spoke to my client. **X8 Therapist:** I did not. He declined to attend. I wrote to him once, at his wife's request, offering an appointment, and I have the letter in my file. — The fact, plus the document, plus the correction of the implication that the therapist excluded him. **X9 Counsel:** Would it not have been better to insist? **X10 Therapist:** I cannot compel an adult to attend therapy. (pause) I can record that I invited him. — The answer to a "would it not have been better" question, which is counsel's favourite form: it invites the witness to criticise herself. The witness states what was possible. **X11 Counsel:** You recorded that she left the children with a neighbour overnight. That is a safeguarding matter, is it not? **X12 Therapist:** I recorded what she told me. I did not treat it as a safeguarding concern, because it was one night, with a known neighbour, and she told me herself without being asked. (pause) If I had thought a child was at risk I would have reported it, and I would have recorded that too. — The factual answer, the reasoning, and the statement of the duty — three elements in three sentences. **X13 Counsel:** With respect, that is your judgement. **X14 Therapist:** It is. And it is the judgement I am employed to make, and it is written down with the reasons. — The final turn of the transcript, and the one students should memorise: the witness owns the judgement, states its basis, and does not apologise for having made it.

What the transcript shows. The therapist's longest answer is forty words and most are under fifteen. She concedes what is true, corrects what is false, and never argues. The single most damaging thing that could have happened in that box was a therapist who sounded as though she were the client's advocate — and the discipline that prevented it was grammatical: she spoke about what she observed, in the past tense, and left the conclusions to the court.

11.8 Laboratory: the note lab and the subpoena drill

Part A — the note lab (thirty minutes). Students are given a ten-minute transcript of a family session containing: a risk disclosure, an individual aside, an argument between the parents, and a plan. Each writes the note in eight minutes. The notes are exchanged and marked against the "three readers" standard by a partner, who must answer three questions from the note alone: what is the risk, what was decided, and what would I do if the client telephoned tonight?

Criterion	0–1	2–3	4–5
Fact and register	Judgements mixed with facts	Mostly factual	Every statement in the correct register
Risk	Not recorded	Recorded as a statement	Questions asked, answers given, level and plan recorded

Criterion	0–1	2–3	4–5
The individual aside	In the main note	Omitted	In the restricted section, with the reason recorded
Plan and consent	Absent	General	Specific, dated, with who was told what and the consent
Handover	Unusable	Adequate	A colleague could take over tonight

Part B — the subpoena drill (thirty minutes). Each student receives one of four documents: a subpoena duces tecum, a letter from a solicitor requesting records "as a matter of urgency", a request from a school for "the records of the family", and a request from a client's employer. In twelve minutes they must write: the first five actions, the letter or response, and the file entry. The group then compares the four responses, which is the point of the exercise — the same instinct produces four different correct answers.

The four correct answers, in summary. Subpoena: verify, tell the client, take advice, produce what is required, log. Solicitor's letter: this is a request, not an order; consent or a court order is required; reply in writing. School: attendance only, with consent, unless a safeguarding duty requires more. Employer: attendance only, with consent, and a note explaining the limit to the employer.

Marks for Part B (10). Correct identification of the instrument; the client informed; advice taken; minimum necessary applied; the file entry complete.

11.8a The four-minute handover note

The note that matters most is the one written when a therapist goes on leave, becomes ill, or leaves a post. It is four minutes' work and almost never done.

The four lines per case.

1. **Who and why** — the client system, the presenting problem, and the agreed focus.
2. **The current risk** — the last assessment, the date, the plan, and who knows.
3. **The live issues** — the secret being held, the referral pending, the court date, the person who has not been told something.
4. **What to do first** — the call to make, the appointment to keep, the file to read.

What belongs at the top of the bundle. A single page listing every client with a risk flag, a court date, or an open protection matter, in date order. It is the page a covering colleague reads on the first evening.

Why it is an ethical duty and not an administrative one. Continuity of care is a client's interest. The therapist who leaves without a handover has transferred her caseload and abandoned its clinical responsibility, and the case in which this matters most is always the one with the highest risk — because that is the case with the most to record and the greatest reluctance to reduce it to four lines.

11.9 Six errors

Error 1 — The note that records a conclusion without its basis. "Client is depressed." The next clinician cannot tell whether this was an assessment, a diagnosis, or an impression, and neither can a court.

Error 2 — The individual disclosure in the joint file. It is the commonest documentary failure in couple work and it is created by an administrative habit rather than a decision.

Error 3 — Promising that the file can never be disclosed. It is false, it is said to reassure, and it converts a lawful disclosure into a betrayal.

Error 4 — Producing the entire file in response to a narrow order. It breaches duties owed to third parties and to the other partner, and it is usually done out of anxiety rather than necessity.

Error 5 — Altering or destroying records. It is a criminal offence in most jurisdictions, it is detectable, and it converts a defensible case into an indefensible one.

Error 6 — Advocacy in the witness box. The therapist who argues for the client's case has abandoned the only thing that made her evidence useful, which is that she was not a partisan.

11.9a Ten sentences that have been read back to a therapist in a hearing

Each of these was written in a file and each was quoted. They are collected because the pattern is instructive: none is a lie, and each is an assertion that the therapist could not support when asked.

1. "Client is manipulative." Question asked: "On what evidence?"
2. "Mother appears to prefer the son." Question: "Which sessions, and what did you observe?"
3. "The marriage is characterised by his control." Question: "You never met him, did you?"
4. "Client is not at risk." Question: "How could you know that?"
5. "She is a devoted mother." Question: "That is an opinion, is it not?"
6. "Father is in denial." Question: "Did you assess him?"
7. "Client has borderline traits." Question: "Are you qualified to make that diagnosis?"
8. "The family is enmeshed." Question: "What does that mean in observable terms?"
9. "Client was intoxicated at session." Question: "Was this tested, or observed, and by whom?"
10. "I am certain the disclosure is true." Question: "Certain on what basis?"

The rule that produces all ten errors. The sentence was written to capture a feeling, in a document that requires evidence. The remedy is not fewer opinions but labelled ones: "I observed X. My impression, which I have not tested, is Y."

11.10 Culture, faith and the record

"The family wants to see what you have written." In households where information is held collectively, the idea that a therapist keeps a private record is foreign and suspicious. The request may come from an elder, a husband, or a client herself who has been sent to fetch it. The answer is not a lecture on data protection. It is three sentences: what the record is (the therapist's working note), who may see it (the client, and a court if it orders), and what the therapist will do instead (summarise in the session what she has written, and let the client correct anything inaccurate).

The client's right to her own record. Where a client asks to see what is written about her, the request is usually legitimate and should not be treated as hostility. Reading the record with the client, in a session, is frequently therapeutic: it demonstrates that the therapist has understood, it corrects errors, and it makes the record collaborative rather than mysterious.

Privacy in a shared house. A record kept on a phone in a household where the husband checks the phone is not private. This is an argument for paper in a locked cabinet, for a practice system rather than a personal device, and for the practice policy that nothing clinical is stored on a personal phone. In the settings this book is written for, that is not a technicality; it is a safety matter.

Confession, records and religious authority. A religious leader who asks to see the therapist's notes about a member of the congregation is making a request that has no legal basis and considerable social force. The answer is the same as for any third party, delivered with respect: "I cannot discuss her with you. I can tell you that she is being seen, if she has agreed to that."

The family's own record. Many families keep their own documentation — letters, court papers, a written account of events. Where the therapist is shown it, the fact is recorded, because the file must show the source of information that reached the treatment from outside, and because the document may later be produced by the family and attributed to the therapy.

> **DEBATE**
> "A therapist's notes should be written on the assumption that the client will read them." Argue for: transparency, accuracy, the client's right of access, and the discipline it imposes. Argue against: the clinical record must contain hypotheses and observations not intended for the client, candour would be lost, and the therapeutic relationship would change. Then state your practice, and the note you would write differently if the debate changed your mind.

11.10a The retention, storage and destruction schedule

Three decisions that every practice must make in advance, because each of them is impossible to make well at the moment it arises.

Retention. How long is a record kept after the last contact? The answer is set by the professional body, by limitation periods for claims, and by the needs of former clients who

may return. In most jurisdictions the practical range for adult records is seven to ten years, and for children's records until some years after the child's majority. The rule should be written, applied consistently, and stated in the privacy notice the client receives.

Storage. Who can reach the record; what happens if the therapist is ill, dies, or leaves the country; and where the backup lives. A practice of one is a practice with a single point of failure, and the "professional will" — a named colleague who can access records, contact clients and close the practice — is part of the duty to clients rather than an act of foresight.

Destruction. How, by whom, and with what record. A file that is destroyed is recorded as destroyed, with the date and the method, and the destruction is subject to a check that no request or proceeding is pending. The practitioner who shreds a file during a complaint has committed the offence described in Error 5.

The client's information. What the client is told at the start: what will be kept, for how long, who may see it, what happens on the therapist's death or retirement, and how the client can obtain a copy. Three sentences in the consent conversation, and a paragraph in the written information.

11.11 Evidence note

What is known. Studies of clinical records consistently find high rates of missing information on risk, consent and reasoning, and that record quality is improved by structured templates and by audit with feedback. Research on client access to records finds that reading them is usually experienced as neutral or positive and rarely harmful. In the forensic literature, the quality of the contemporaneous record is one of the strongest determinants of how a clinician's conduct is judged in a complaint or a hearing.

What is weaker. There is little research on the specific formats used in family therapy, and none on the restricted-section practice recommended here, which is derived from the structure of multiperson confidentiality rather than from outcome evidence.

What is not supported. That detailed notes protect a therapist regardless of content — a detailed note containing judgements is more dangerous than a sparse factual one. That the client's access to records is generally harmful. And that destroying or amending records ever improves a position.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same sentence, written in a supervision note and written in the clinical record, has two different lives in a courtroom.

11.12 Key terms

Contemporaneous note — a record made at or near the time of the event; its evidential weight is much greater.

Restricted section — a part of a couple or family file holding material that must not be disclosed to another client in the same case.

Privilege — the client's legal protection against disclosure in proceedings; qualified and waivable.

Subpoena duces tecum — an order to produce documents; distinct from a request by a party.

Redaction — the removal of specified information from a disclosed document, recorded with reasons.

Witness of fact — a witness reporting what she observed; contrasted with an expert witness offering opinion.

Minimum necessary — the principle that disclosure is limited to what the purpose requires.

11.12a The two-minute note review

A habit rather than a technique, and the cheapest quality improvement available: at the end of each working day, the therapist re-reads the notes she wrote and asks four questions of each.

1. Could a colleague act on this tonight?
2. Have I written a judgement where I should have written an observation?
3. Have I recorded the risk questions I asked, or only the answers?
4. Is there anything here I would not want read aloud?

Ten notes take two minutes and question 4 is the one that changes practice. A therapist who asks it daily stops writing the sentences that get read back in a hearing (section 11.9a) within about three weeks.

11.13 Self-test

1. Name the three readers of a clinical note and what each needs.
2. Give the four registers, with an example of each.
3. State the six headings of a family session note.
4. What is the restricted section, and what three clauses describe it?
5. Give the five checks to perform in the first hour after a subpoena arrives.
6. Distinguish confidentiality from privilege.
7. Name the four situations in which privilege will not protect the file.
8. Give the four rules for giving evidence, and explain why "I do not know" is a strong answer.

Answers. (1) The therapist in six months — problem, attempts, outcome, plan; a colleague taking over — risk, plan, who knows, what to do if telephoned; a court — facts with dates, decisions with reasons, consent, no speculation. (2) Fact: "client attended on 4 March"; observation: "she checked her phone four times"; opinion: "consistent with an acute stress response"; hypothesis: "possible that the husband's drinking is a factor, not yet explored". (3) Attendees; presenting this session; risk screen; interventions; response; plan. (4) A

clearly marked part of a couple or family file holding individual-session material that must not be shared with the other client; described by its reference to practice policy, the client's knowledge that it will not be shared, and the statement that nothing affecting safety is withheld from the main file. (5) Validity and addressee; exact scope; whether the client has been told; who is advising; the deadline and whether there is time to object. (6) Confidentiality is the therapist's professional duty; privilege is the client's legal protection, waivable by the client and overridable by statute or court order. (7) Client waiver, frequently by putting mental state in issue; a statutory exception such as child protection; a court order; and proceedings in which the therapist is defending herself. (8) Answer the question asked; do not guess; distinguish fact from opinion; do not defend the treatment. "I do not know" is strong because it is credible, it prevents the improvisation that destroys witnesses, and no court expects a clinician to know everything.

11.14 Reflective task

Write one page on the following before you read further: which of the four registers do you default to when you are tired, and what does that default produce in a record that might one day be read aloud? Most therapists default to opinion, because opinion is what the mind produces when it is summarising quickly, and opinion is the least defensible thing in a file.

Take the last note you wrote in a placement or practice and read it as three people: as the therapist in six months, as a colleague on call tonight, and as counsel cross-examining you. Write a paragraph on what each of them would make of it. Then rewrite one sentence.

11.15 Further reading

- Any current text on record keeping and the legal process in health and social care in your jurisdiction.
- The Nigeria Data Protection Act 2023, and your professional body's guidance on records and access.
- Gutheil, T. on the psychiatrist in court — despite the discipline, the clearest treatment of testifying as a clinician.
- Your indemnity provider's guidance on subpoenas and on responding to requests for records — usually free and usually unread.
- Court rules on the production of documents in family proceedings where you practise.

Next: Week 12 — Money, gifts and the economics of the relationship.

Week 12 — Money, gifts and the business of therapy

Learning outcomes. By the end of this week the student can (1) explain why the fee is clinical material and not an administrative detail; (2) conduct the fee conversation at intake, including the sliding scale and the cancellation policy, in five minutes; (3) manage non-payment clinically rather than avoiding it; (4) apply a decision procedure to any gift and record the outcome; and (5) identify the business pressures that produce unethical practice and state the limits that protect the client.

The subject nobody teaches and everybody practises

Money is the least discussed and most consistently present element of the therapeutic relationship. It is present at the first contact, in every session, in the waiting room, in the client's calculation of whether she can afford to continue, and in the therapist's calculation of whether she can afford to keep her. It carries meaning about worth, entitlement, dependency, shame, power and care — and because therapists are frequently uncomfortable with money, the meaning is usually not discussed at all.

The three-part claim of this week. First, the fee is a clinical instrument: it defines the frame, it makes the relationship professional, and it is one of the ways in which the client's difficulties with entitlement, need and worth appear in the room. Second, the therapist's own relationship with money is a source of error, and it is rarely examined. Third, a practice that is not financially viable will exploit its clients in ways that are rarely deliberate and always structural.

Situation	Clinical meaning, usually	Ethical requirement
A client who always pays late	Helplessness, resentment, a test of whether the therapist will chase	A stated policy, applied consistently, discussed as material
A client who pays in advance for ten sessions	Fear of abandonment; an attempt to secure the therapist	Discuss what the payment means before accepting it
A client who asks for a reduction	Shame; a belief that she is not worth the full fee	A written policy, a decision made once, recorded
A therapist who never chases a debt	The therapist's own discomfort with conflict or with greed	Supervision; a policy applied by the practice rather than the person
A gift after a good outcome	Gratitude; a wish to be remembered; obligation	A decision procedure, a conversation, a record
A gift after a disclosure of shame	An attempt to repair the therapist's view of her; a payment	Often to be refused, with the meaning named

12.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	Money as clinical material: three vignettes, and the therapist's discomfort	Anonymous written answers to "what did your family teach you about money?"
10–22	Five concepts: frame, transference, the therapist's money, sliding scale, viability	Lecture

Minutes	Content	Teaching form
22–38	The intake fee conversation, scripted and rehearsed	Demonstration, then triads
38–52	Non-payment: the sequence, the limit, the ending	Case, with students writing the sentence for month two
52–68	Gifts: the four categories and the decision procedure	Card sort of eight gifts; then the refusal script
68–80	The business pressures: discounts, packages, marketing, billing	Lecture with four examples, students identify the risk
80–90	The Nwankwo case	Case, students decide what is accepted, what is charged and what is said

12.2 Five concepts that organise the week

Concept 1 — The fee is part of the frame, and the frame is the treatment. A therapy has an agreed time, place, frequency, duration and payment. The payment is not the least of these: it is what makes the relationship a professional one rather than a friendship, and it is what allows the client to feel entitled to the therapist's attention. A client who is not paying, or who is paying unpredictably, is frequently a client who does not feel entitled to ask for anything.

Concept 2 — Money carries the client's history, and the therapist should expect it. The client who cannot ask for a reduction and stops attending; the client who insists on paying for the whole course in advance; the client who gives an expensive gift and then asks for a favour; the client who treats the fee as a debt of gratitude. Each is a repetition of something, and each becomes treatable when it is named.

Concept 3 — The therapist's own money is the largest unexamined variable in the room. Therapists under-charge because they are anxious about being seen as commercial, or because they do not believe their work is worth the fee, or because a client's poverty activates guilt. They over-charge, or cling to clients they should refer, when the practice is not viable. Neither is a moral failing; both are material for supervision, and neither is correctable by willpower.

Concept 4 — The sliding scale is an instrument of justice and a machine for resentment. A scale that is written, published internally, applied by rule and reviewed annually serves clients and protects the therapist. A scale that is negotiated case by case, in the room, according to how sorry the therapist feels, produces inconsistency, the client's discovery of another client's lower fee, and a therapist who is quietly furious.

Concept 5 — Viability is an ethical matter. A practice that cannot pay its rent will raise its fees abruptly, take clients it should refer, and cut the supervision that protects clients. The ethical practice is a sustainable one: priced to survive, with the pro bono work built into the budget rather than subtracted from it at the end of a difficult month.

12.2a The intake fee conversation, in five minutes

Six statements, made at the first contact, in the order below, and repeated in writing.

1. **The fee, plainly.** "My fee is X per session, and sessions are fifty minutes." No apology, no explanation of overheads, no "I know it is expensive".
2. **What the fee includes, and what it does not.** The session; correspondence with other professionals where consented; a written report where one is agreed. Not telephone calls between sessions, which are handled by agreement.
3. **When and how it is paid.** "Payment is due at the end of each session, by transfer or in cash. I do not carry change." The mechanics are stated because ambiguity about mechanics is where resentment begins.
4. **Cancellations.** "I require twenty-four hours' notice. If you cancel with less, or do not attend, the session is charged, because the time was held for you." Then the sentence that makes it acceptable: "if you are ill or there is an emergency, tell me and I will not charge you."
5. **The sliding scale, if there is one.** "I have a small number of reduced-fee places. If you need one, tell me now or at any point, and I will tell you honestly whether one is available." The phrase "tell me honestly whether one is available" is important: it makes the scale a resource rather than a favour.
6. **What happens if payment becomes difficult.** "If you ever cannot pay, tell me early. We will find a way forward, and I would much rather know than discover it." Said at the start, this sentence makes the later conversation possible.

A worked fee dialogue.

> **Client:** How much do you charge? **Therapist:** X per session. Sessions are fifty minutes, and payment is at the end of each session. **Client:** (pause) That is — I am a teacher. **Therapist:** Then let us talk about it now rather than later. I have two reduced places at the moment; one is taken. (pause) What would be manageable for you? **Client:** I do not know. I do not want to be a charity case. **Therapist:** You are not. The reduced places exist because I would rather work with a full practice than an expensive empty one. (pause) If we agree a figure, we will review it in six months, and if your situation changes we will change it then.

Why the last sentence matters. A reduced fee without a review becomes permanent by default, and both parties know it. A stated review makes the arrangement a professional one and gives the client a way to increase the fee without having to raise it.

The three questions the therapist should be able to answer without flinching. What is my fee? Why is it that? And what will I do if this client cannot pay it? A therapist who cannot answer the second has not priced her work; she has inherited a number.

12.2b Non-payment: the sequence and the limit

The clinical problem. Non-payment is rarely about money alone. It is frequently about shame (the client who cannot say she cannot pay), about entitlement (the client who believes the therapist owes her), about a test (will you chase me, and does my worth depend on your not asking?), or about a genuine catastrophe. The therapist who chases aggressively and the therapist who never raises it both fail; the first humiliates, the second colludes and then resents.

The sequence, session by session.

At the first missed payment: ask, in the session, without accusation. "I noticed the last two sessions were not paid. Is something happening?" The question is about the client, not the debt.

At the second: name the pattern and the feeling. "This is the second month. I want to say what I notice, which is that you have not mentioned it, and I wonder whether it is difficult to talk about." Then the agreement: a date, an amount, and whether the sessions continue meanwhile.

At the third: the limit. "I am going to be honest with you. I cannot continue to hold sessions that are not paid, and I am not willing to end the therapy because of money without us having tried to solve it. Here are the options: a reduced fee for a period, a payment plan, a pause, or a referral to a service that is free."

At the fourth: the decision, made and recorded. Continuing indefinitely is not generosity; it is a slow resentment that the client will feel, and it ends with the therapist withdrawing at the worst possible moment.

The sentences that work, and the ones that do not.

Say	Not
"I need to raise something about the account."	"You have not paid."
"I would rather we talked about it than that I sent a letter."	"This is a business, you know."
"What would be manageable?"	"I have been very patient."
"If we cannot agree, I will refer you and we will end properly."	"I cannot help people who do not pay."

The clinical gain in the conversation. A client who has never been able to negotiate a debt with anyone — and this is common in clients whose financial lives are chaotic or controlled — has the experience of a relationship in which a difficulty is named, negotiated and survived. That is often worth more than the fee.

Ending for non-payment, done properly. A closing session, a written summary, a referral, an offer to return if circumstances change, and a record of the reasoning. Not a letter from an agency, and not silence.

12.2c The therapist who is owed money, and cannot say so

The countertransference of the unpaid fee is one of the most reliable in practice and one of the least discussed.

The four patterns.

1. **The rescuer.** The therapist decides that the client's poverty is part of the problem and that charging is therefore cruel. Sessions continue, the fee is never discussed, and the therapist begins to feel quietly exploited. The client, sensing it, apologises for existing.

2. **The withdrawing therapist.** The therapist begins to offer shorter sessions, fewer appointments, less attention. The client experiences a loss of interest and cannot say why.
3. **The punitive therapist.** The debt becomes a moral matter. The therapist's tone becomes cool, and the client — who has usually been punished before — recognises the tone before she can name it.
4. **The avoider.** The therapist genuinely does not think about the debt until the accountant raises it, and then sends a letter written in a completely different voice from the one used in the room.

The examination. Before the conversation, three questions in writing: what do I feel about being owed this money? What do I imagine the client feels? And what am I doing differently in the sessions since the debt began? The third question is usually answered with a specific change — the therapist has stopped challenging, or has started ending on time with unusual firmness — and that change is the clinical material.

The rule. The therapist's discomfort must not be paid for by the client, in either direction: neither by waiving the fee so that the therapist does not have to have the conversation, nor by extracting the money in a way that punishes her. The conversation is the only way through, and it is the therapist's to initiate.

12.3 Gifts: four categories and one procedure

Category 1 — the cultural token. Food, cloth, a small item given at a festival or a home visit. Value is modest, the meaning is relational, and refusing it can be experienced as rejection. Default: accept, name the meaning, record it, and do not treat it as a transaction.

Category 2 — the transitional gift. Given at the end of treatment, at a birth, at a bereavement, or after a significant improvement. Default: usually accept if modest; discuss what it means; record it. The discussion frequently produces the most important session of the final month.

Category 3 — the expensive gift. Value beyond a token; jewellery, electronics, a large sum, land. Default: decline, warmly, with an explanation that protects the giver's dignity: "I am very moved that you would do this, and I am not able to accept it. It would change something between us that I need to keep as it is."

Category 4 — the gift with strings. Any gift accompanied by a request, an expectation, or a narrative about what the therapist now owes. Default: decline, and name the string, because a gift that creates an obligation has already begun to distort the relationship.

The procedure, in five steps.

1. **Notice.** Register the gift as a clinical event rather than an administrative one, and do not decide in the

doorway.

1. **Ask what it means.** "What would it mean to you if I accepted this?" and, where relevant, "and what would it mean if I did not?" The answers are almost always more informative than the object.
2. **Decide against the category,** not against the person's need. The therapist who accepts because the client will be hurt has made the client's reaction the criterion.
3. **Say the decision, once, warmly, and without a lecture.** The lecture ("you know, ethically, I cannot") turns a gift into an embarrassment.
4. **Record it.** One line: what was given, what was decided, what was discussed, and what happened to the object.

The gift that must be refused, whatever its value. A gift from a client who is in a coercive relationship and who is spending money her partner controls, because accepting it colludes in the control. A gift from a child client that has clearly been taken from a parent's belongings. A gift that is the client's only asset. And a gift given immediately after a disclosure of shame, because the therapist is being paid to stop seeing something.

The gift that is a communication. In many of these cases the gift says something the client cannot: "I am not only a problem", "you have been good to my mother", "I want you to remember me". Naming the communication — "I think this says something about how you want me to see you" — frequently makes the object unimportant, and the client may then be content for the therapist to keep it or return it.

What to do with what is accepted. Food is shared or eaten; cloth may be worn or kept; money is never accepted. Where the practice has a policy that accepted gifts become the property of the clinician, the policy is stated in writing at intake, which prevents the later argument about whether the therapist "kept" something.

12.3a Barter, pro bono, and the client who cannot pay at all

Barter. The exchange of goods or services for treatment. Permissible in narrow conditions: the client initiates; the value is agreed in writing; the arrangement is time-limited; it is supervised; and the therapist records the rationale. Prohibited in practice where the client's service is intimate, where the value is uncertain, or where the exchange would place the therapist in a position of judging the client's work.

The specific hazard of barter. The client's service becomes clinical material. The therapist who accepts tailoring from a client will have views about the tailoring, and the client will know it. What was a fee becomes a performance, and the therapy acquires a second arena in which the client can fail.

Pro bono. Treatment given without fee. It is a professional obligation in most codes, in some form, and it is best organised as a proportion of the practice — one or two places held open — rather than as an occasional decision made under pressure in the room. The client on a pro bono place is told that it is a place, not a favour, and that the arrangement is reviewed annually. That framing protects both parties from the gratitude that otherwise contaminates the work.

The client who cannot pay at all. Three options, in order of preference. A reduced or waived fee for a defined period, with a review. A referral to a free service, with a warm

handover. A short piece of work — six sessions — focused on a single question, rather than an open-ended therapy that will collapse in month three.

The option that must be avoided. Continuing indefinitely without agreement, because the therapist cannot bear to raise it. The client will sense the unspoken arrangement, will begin to apologise for existing, and will leave without a closing. The ending that results is worse for her than the one the therapist was avoiding.

The practitioner's own arithmetic. A practice with four reduced places, each at half fee, has given away two full fees a week. That is a real contribution and it must be in the budget. The practitioner who gives away places ad hoc and then raises everyone's fee to compensate has made the poorest clients pay for the therapist's generosity to other clients.

12.3b The gift from the family, not the client

A distinct problem, and common in family work: the gift comes from a member of the family who is not the identified client — the grandmother, the uncle who pays, the child.

Why it is different. The giver may not be in treatment; the meaning may be about the family's hierarchy rather than about gratitude to the therapist; and accepting from one member can create a coalition that the other members can see.

The procedure. Treat it as the gift from a client, and add two steps. Establish who decided that the gift would be given, because a gift offered by a father on behalf of a family in which the mother is silent is a statement about who speaks. And where the giver is not a client, decline more readily: there is no therapeutic relationship to preserve, and the gift is frequently an attempt to establish one.

The child's gift. A drawing, a card, a stone from the school playground: accept it, thank the child, and where the child is in a family treatment, keep it visible for a session. A child who gives a gift is testing whether the therapist is a person who can be given to, and the answer matters clinically.

12.4 The business: eight pressures that produce unethical practice

1. The package discount. "Ten sessions for the price of eight." It is a commercial instrument in a clinical relationship, it creates pressure on the client to continue when she would have stopped, and it creates pressure on the therapist to keep her coming. Where a package is offered it must be refundable, optional, and never presented as a saving against distress.

2. The productivity target. An employer who requires a fixed number of clinical hours per week, or a fixed income from each clinician, creates an incentive to keep clients in treatment, to shorten sessions, or to accept clients outside competence. The individual therapist cannot change the system; she can record the pressure and refuse the three specific practices that harm clients — retention for revenue, treatment without supervision, and records written to satisfy a billing code.

3. Upcoding and creative billing. Describing a session in terms that maximise reimbursement, or recording a diagnosis that the client does not meet in order to satisfy

an insurer. This is fraud, it is a disciplinary matter, and it also corrupts the clinical record — the diagnosis written for the payer becomes the diagnosis that shapes the treatment.

4. The referral arrangement. A payment, a commission or a reciprocal arrangement for referrals. In most jurisdictions it is prohibited, and in all of them it corrupts the one decision the client depends on: that the referral was made in her interest.

5. Testimonials and advertising. Client testimonials are prohibited or restricted in most codes, because consent to treatment is not consent to be advertised and because the testimonial is a disclosure. Claims of effectiveness that cannot be supported — "guaranteed results", "the only therapist who..." — are misleading conduct, regardless of the therapist's sincerity.

6. The product sold to clients. Books, courses, supplements, retreats. There is nothing inherently wrong with a therapist writing a book; there is a serious problem with recommending to a current client a product from which the therapist profits, without disclosure and without an alternative.

7. The consultancy overlap. The therapist who is engaged by an employer to advise about an employee who is also her client. The two roles have different clients and different duties; the conflict is structural and should be declined rather than managed.

8. The unpaid work that is expected. Reports for courts, letters for schools, telephone calls with social workers: frequently unbilled and frequently essential. A practice that does not price this work either does it resentfully or refuses it at the moment the client most needs it. The remedy is a stated fee for reports and correspondence, discussed at intake (12.2a), and applied without embarrassment.

12.4a The practice that survives without exploiting anyone

Five arrangements, each of which is a decision made in advance rather than in the moment.

Price for survival. The fee is set to cover the practice's costs, the therapist's income, supervision, insurance, continuing development and a margin for the unpaid work. A fee set to be "affordable" by an anxious therapist who has not done the arithmetic becomes a fee that rises unpredictably in year three.

Build the pro bono into the plan. One or two places, budgeted, reviewed annually, publicly described in the practice's information as part of how it works.

Charge for the work that is work. Reports, court attendance, extended telephone calls and meetings with other professionals have stated fees. The client is told in advance; the alternative is a therapist who resents the letter she is writing at ten at night.

Hold a no-show policy that is fair and enforced. Twenty-four hours, with a stated exception for emergency and illness, applied consistently, and discussed clinically when it recurs.

Keep supervision in the budget. It is the last item most practices cut and the first that protects clients. A practice that cannot afford supervision cannot afford to practise.

The annual review. Once a year: what is the fee, what does it cover, what is the pro bono allocation, what is the debt, and what did I do last year that I would not defend to a colleague? The review is the practice's equivalent of the record of consent — the evidence that the arrangements were chosen.

12.4b What the client is told, in writing

The money policy belongs in the written information the client receives at intake, in one page and in plain language. Six paragraphs.

The fee. The amount, the session length, and what it covers. **Payment.** When it is due, and how it is made. **Cancellations.** The notice required, the charge, and the exceptions. **The sliding scale.** That places exist, that they are limited, how they are allocated, and that they are reviewed. **Difficulties.** That the client should tell the therapist early if payment becomes difficult, and that the therapist would rather know than discover it. **Records and confidentiality.** A cross-reference to the consent document, and the statement that the record may be seen by the client and may be compelled by a court.

Why the written page matters more than the spoken conversation. The client under stress remembers about one-third of what is said in a first session. The page is read at home, sometimes twice, and frequently by a partner — which is itself a disclosure the therapist should have anticipated and stated: "you may show this page to whoever you wish; it contains nothing about your treatment."

12.5 Case: the Nwankwos, the fee, the goat and the plot

The family. Chuka Nwankwo, 44, a civil engineer; Ifeoma Nwankwo, 40, a trader in fabric; children 14 and 9. They were referred by their physician after the death of Ifeoma's mother, with whom the household had lived.

The first four months. Weekly couple sessions, then fortnightly. The presenting problem was the household's grief and the conflict between Ifeoma and her husband's sister, who had moved in "to help" and stayed. The work went well by every measure that matters: the conflict reduced, the children were brought for three sessions, and the grief was named.

Month five: the job. Chuka was told his contract would not be renewed. The family's income halved. In session eighteen he said, at the end, "we may need to reduce the sessions". The therapist's response was the sentence she had practised at intake: "thank you for telling me now rather than in three months. Let us talk about what is manageable."

The agreement. Sessions fortnightly rather than weekly; the fee reduced by forty per cent for six months; a review in month eleven. Recorded in the note and in a short letter to the family. The therapist's own note recorded the reason and the review date, which is the difference between a sliding scale and a favour.

Month seven: the goat. Ifeoma arrived with a live goat, tied, on the back of a neighbour's pickup, "for the therapist's family, because of what you did for my mother before she died". The therapist's first instinct was refusal — a live animal in a city clinic — and her second was to accept immediately so as not to shame her.

What she did. She neither accepted nor refused in the doorway. She said: "this is the most generous thing anyone has brought me, and I want to talk about it properly. Can we sit down for five minutes?" Then she asked what the gift meant. Ifeoma's answer: "in my place, when somebody does something for your mother, you do not say thank you with words."

The therapist then made a decision on the category rather than on the feeling: a live animal is not a token and its value is not trivial, and it also cannot be returned without humiliation. She proposed a third option: "I will accept it, and I will not take it home. I will ask you to sell it and to put the money towards the remaining fee — which means you have paid me, in the way your mother would have understood."

Ifeoma refused, then accepted, then cried, and the transaction became the subject of the best session of the treatment: a family that had been unable to receive help without paying for it, and a therapist who had found a way to let them pay.

Month eleven: the plot. Chuka's contract was renewed. He attended alone and said that a relation had given the family a plot of land on the outskirts of the city, and that they wanted to transfer it to the therapist "as a thanks, because the marriage would have ended".

The decision. Declined, immediately and unambiguously, with the explanation that protects: "I cannot accept this. It is worth more than the whole of the treatment, and if I took it, you would always know that I had, and I would always know it too. (pause) What I would like, if you want to mark this, is a letter. Not for the file, for me."

The letter. Chuka wrote two pages. It is not in the file, it is not evidence, and it was, the therapist later said, the only gift in twenty years of practice that she had wanted.

What the case teaches.

1. **The intake sentence made month five possible.** "If you ever cannot pay, tell me early" was said at the start; without it, Chuka would have cancelled the sessions without explanation and the treatment would have ended silently.
2. **The decision was made on the category, not on the feeling.** The goat was a live animal of nontrivial value; the plot was an asset. The therapist's discomfort and the family's generosity were both real and neither was the criterion.
3. **The third option is usually available.** Accepting and refusing were not the only choices; "sell it and put the money towards the fee" preserved the meaning, protected the frame and gave the family a way to pay that their own culture recognised.
4. **The conversation about the gift was the therapy.** Both gifts arrived at moments when the family was working out whether help could be received without debt — which was, in retrospect, the family's central problem and the reason the sister-in-law had stayed for two years.
5. **The refusal of the plot was the more generous act.** The therapist who accepted it would have been, from that day, a person to whom the family owed everything, and the therapy would have ended in a way nobody could describe.

12.6 Transcript: the fee conversation at month two of non-payment

A different case, from the therapist's own practice log. Mrs Bello, 33, has attended eight sessions and has paid for three. Minutes twelve to twenty-four.

> **F1 Therapist:** I want to raise something and I have been putting it off, which I am telling you because it is true. — The confession of reluctance, which disarms the accusation before it lands. **F2 Client:** The money. **F3 Therapist:** The money. (pause) Three sessions have been paid for and eight have happened. — The facts, without an adjective. "Eight have happened" rather than "you owe me for five" — the same information, without the debt-collector's grammar. **F4 Client:** I am sorry. I am so sorry. I will bring it next week. **F5 Therapist:** (pause) You have said that twice. (pause) I am not asking for an apology, and I am not going to pretend I have not noticed. — The pattern named, gently, and the apology declined, because the apology is the thing that prevents the conversation. **F6 Client:** (long pause) My husband controls the account. **F7 Therapist:** (pause) Tell me about that. — Three words. The fee conversation has just become the clinical material, which is what the concept in 12.2 predicted and what the therapist was waiting for. **F8 Client:** He gives me money for the house. If I ask for more he wants to know what for. He does not know I am coming here. **F9 Therapist:** So every session you have attended has been paid for out of — what? **F10 Client:** Food money. I take a little. **F11 Therapist:** (pause) So you have been buying your therapy with your children's food money, and you have not been able to tell me. — The reflection that names the cost without moralising. The client's face, recorded in the note, changed. **F12 Client:** (crying) I did not want you to think I was like that. **F13 Therapist:** Like what? **F14 Client:** Somebody who does not pay. **F15 Therapist:** (pause) We are going to change this today, and it is not going to be you paying more. (pause) First, I want to know whether coming here is safe for you. — The pivot that matters: the fee has revealed a safety question, and the question takes priority over the account.

What the transcript shows. The therapist raised the money, named the pattern, declined the apology, and then followed the material wherever it went. Within eleven minutes of the fee conversation, the case had become a case about financial control — which had been in the room for eight sessions and had not been visible because nobody had mentioned the money.

The outcome. The fee was waived for three months while a safety plan was made; the client's attendance was recorded as "no charge, agreed for safety reasons, reviewed monthly"; and the financial control became the central subject of the treatment. The therapist also changed her intake: the question "who knows that you are coming here?" now appears in the first session of every case.

12.6a The five sentences of the money conversation

Students ask for a script, and a script of five sentences is more useful than a policy document.

1. **"I want to raise something about money, and I have been putting it off."** Every difficult conversation in this chapter begins with the therapist admitting the reluctance, which lowers the client's fear of being judged.

2. **"The facts are these."** The number of sessions, the number paid, the amount outstanding. No adjectives.
3. **"What is happening?"** A question about the client's situation rather than about her conduct, which is where the clinical material lives.
4. **"Here is what I can do, and here is what I cannot."** The options, stated as the therapist's capacity rather than as the client's failure.
5. **"Whatever we agree, we will write it down and we will look at it again on [date]."** The date that converts an arrangement into a plan.

What the five sentences do not include. An apology for asking, an explanation of the therapist's overheads, an account of how hard the therapist works, or a reassurance that the client is not the first person to do this. Each of those makes the exchange about the therapist.

The rehearsal. Practise the five sentences aloud before the session in which they will be needed. Therapists who have not said them aloud once will soften them in the room, and softening is what produces the ad hoc arrangement that becomes a resentment.

12.7 Laboratory: the fee policy and the gift decision

Part A — write the practice's money policy (thirty minutes). One page, six headings: fee and what it covers; payment mechanics; cancellation; sliding scale and how a place is allocated; pro bono allocation; debt and its limits. Then the group exchanges policies and each student applies another practice's policy to three cases. The exercise reliably produces the discovery that most policies cannot be applied by anyone other than their author, which is the definition of a policy that is really a preference.

Part B — the gift sort (fifteen minutes). Eight gifts, sorted into the four categories of 12.3, with a decision and a sentence for each.

Gift	Category	Likely decision
A bowl of fruit at the end of treatment	Token/transitional	Accept, record
A wrapped watch at session four	Expensive, early	Decline
A live chicken, delivered to the clinic	Token with practical problems	Discuss, propose a third option
A child's drawing	Transitional	Accept, display or keep
Money in an envelope after a good session	Never acceptable	Decline, discuss the meaning
A client's business stock as payment	Barter	Discuss, usually decline
A gift from a client in a controlling marriage	Category four — the string is the control	Decline, and assess safety
The offer to renovate the clinic	Expensive, institutional	Decline, or refer to the practice's owner with disclosure

Part C — the refusal script (ten minutes). Each student writes three sentences for declining a watch, and reads them aloud. The group marks them on two criteria: dignity preserved, and no lecture.

Marks (20 total). Policy: applicable by a stranger (5), covers all six headings (5), states limits (5). Gift decisions: correct category for each and a defensible decision with a sentence (5).

The observation to make in the debrief. The best refusal scripts in the room will be the shortest, and every student will initially write one that explains the ethics of gifts to the client.

12.8 Six errors

Error 1 — Never mentioning the fee again after intake. The client who is struggling has to raise it herself, which the client least able to raise it will not do. The therapist must raise it, and the permission to do so was given at intake.

Error 2 — The ad hoc reduction. A lower fee granted in the room, to a client who asked persuasively, with no review and no record. It is discovered, it is resented, and it is unfair to every client who did not ask.

Error 3 — Chasing a debt by letter. The letter is appropriate at the end of a process, not at the beginning. Sent early, it converts a clinical relationship into a commercial one, and the client who was ashamed becomes frightened.

Error 4 — Accepting an expensive gift to avoid embarrassment. The therapist's discomfort is being paid for with a distortion of the relationship, and the client now holds something over her.

Error 5 — Billing a diagnosis the client does not meet. Fraud, and a corruption of the record that outlives the claim.

Error 6 — Subsidising the practice with the therapist's own unpaid labour, silently. It works for a year, then produces resentment, a sudden fee increase, or a withdrawal from the work that looks to the client like abandonment.

12.9 Culture, faith and the meaning of money

The gift that cannot be refused. In many of the households this book serves, a gift is the language in which a relationship is acknowledged, and its refusal is a statement that the relationship is not accepted. A therapist who declines everything on principle will be described as "proud" and will be told, by the family, that she has "refused their hand". The workable practice — clear in the therapist's own mind and sometimes stated aloud — is that tokens are accepted and assets are not, and that the distinction is explained in the family's terms: "a bowl of fruit is between friends; a plot of land would make me something else."

Payment in kind. Where cash is scarce and goods are not — fabric, food, a service, labour — the family may offer payment in kind without embarrassment. The rules of 12.3a apply, and the cultural reality argues for a narrower rather than a wider permission: the

value is usually uncertain, the arrangement is usually permanent once established, and the therapist's judgement about the client's work contaminates the treatment.

The man who pays. In many households the husband pays for his wife's therapy and will want to know what it is for, which raises the Week 3 problem in a financial form. The practitioner's policy: the fee is agreed with the person who pays, the content is not discussed with anyone, and the arrangement is stated at intake — "your husband will be told that you are attending and what it costs, and nothing else, unless you ask me to tell him more."

The client who cannot pay and the therapist who is a member of the same community. The therapist's own obligations — to a church, a mosque, a family, an association — may include expectations of free help, and refusing them can be costly. The professional answer is a stated policy, applied to everyone: "I have two reduced places and both are taken; I can refer you, or you can wait for a place to come free." Said as a policy rather than as a judgement about the person, it is usually accepted.

Wealth, class and the fee. A therapist from a different class position from her client will misjudge what is affordable in both directions. The correct method is to ask, explicitly, and to believe the answer: "what would be manageable for you?" A client who says two thousand naira means two thousand naira, not a figure that the therapist thinks is realistic.

The religious objection to charging. Some clients, and some religious traditions, regard payment for counsel as improper — "you cannot sell the word of God". The therapist's answer is not a lecture on professional economics but an explanation of what the fee pays for, in one sentence: "the fee pays for the hour you have, the training behind it, and the fact that I can be here next week."

> **DEBATE**
> "Therapists should charge what the market will bear, and give pro bono time as they choose." Argue for: viability is an ethical precondition, the market allocates scarce skill, and discretionary generosity is genuine generosity. Argue against: the market allocates to those who can pay, discretionary charity produces inconsistency and resentment, and a profession that prices by the market has abandoned the poorer half of its obligation. Then set out a fee policy you could defend to both a client who cannot pay and a colleague who thinks you are under-charging.

12.10 Evidence note

What is known. Research on psychotherapy fees is limited but consistent on two points: fee arrangements affect the therapeutic relationship, and therapists' discomfort with discussing fees leads to avoidance which clients experience as distance. Studies of gift-giving in therapy find that gifts are common, that clinicians report uncertainty about them, and that gifts are frequently meaningful communications about the relationship rather than attempts to influence. In medical and psychotherapy ethics, financial conflicts of interest are among the most consistently documented sources of harm, largely through subtle changes in clinical decisions rather than overt fraud.

What is weaker. There is very little research on sliding scales, on barter, or on the outcomes of non-payment policies. The guidance in this week is derived from ethics and from practice rather than from controlled study.

What is not supported. That charging a higher fee produces better outcomes. That accepting a gift always damages the relationship — modest, culturally appropriate gifts are usually experienced as affirming. And that a therapist's financial interest does not affect her clinical judgement; the research on conflicts of interest suggests it reliably does, while the clinician reliably believes it does not.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same reduced fee, agreed at intake and agreed in month nine after three unpaid sessions, is a different arrangement with a different meaning.

12.10a The audit of your own practice

Once a year, five questions, written down, kept with the supervision record.

1. **What is my fee, and what does it pay for?** If the answer cannot be given in one sentence, the fee is inherited rather than set.
2. **How many reduced places do I hold, and how were they allocated?** If the answer is "whoever asked", the scale is a favour.
3. **What did I accept last year, and what did I refuse, and would I defend each decision to a colleague?** The question is not whether the decisions were right but whether they were decisions.
4. **How much did I leave unbilled, and what did that do to my attitude to those clients?** The honest answer is frequently "I began to avoid them", which is the clinical cost of unbilled work.
5. **What did I do for money last year that I would not want a client to know about?** A consultancy, an endorsement, a referral arrangement, a discounted package sold to a distressed family. One question, once a year, and it is the one that keeps a practice honest.

What to do with the answers. Take them to supervision, not to a marketing plan. The audit's purpose is not growth; it is the discovery of the places where financial pressure and clinical judgement have begun to converge.

12.11 Key terms

Frame — the agreed arrangements of time, place, frequency, duration and payment that constitute the treatment.

Sliding scale — a published schedule of reduced fees with stated criteria and review.

Pro bono — treatment provided without fee, budgeted as part of the practice.

Barter — exchange of goods or services for treatment, permitted only in narrow documented conditions.

Conflict of interest — a financial or personal interest that could influence clinical judgement.

Upcoding — describing a service or diagnosis inaccurately to obtain higher reimbursement; fraud.

Testimonial — a client's public statement about a therapist; restricted or prohibited in most codes.

12.12 Self-test

1. Why is the fee clinical material rather than an administrative detail?
2. Give the six statements of the intake fee conversation.
3. State the four-step sequence for non-payment, with the sentence for each stage.
4. Name the four categories of gift and the default for each.
5. Give the five steps of the gift procedure.
6. What are the conditions under which barter may be permissible, and the specific hazard?
7. Name five business pressures that produce unethical practice.
8. In the Nwankwo case, what was the third option for the goat, and why was it better than accepting or refusing?

Answers. (1) Because it constitutes the frame that makes the relationship professional, and because it carries the client's history of entitlement, worth, shame and dependency, which becomes treatable when it is named. (2) The fee and session length; what the fee includes; when and how it is paid; the cancellation policy; the sliding scale; and what happens if payment becomes difficult. (3) First missed payment — "I noticed the last two sessions were not paid; is something happening?"; second — the pattern named, "you have not mentioned it, and I wonder whether it is difficult to talk about"; third — the limit, with four options; fourth — the decision, recorded. (4) Cultural token — accept and record; transitional — usually accept, discuss the meaning; expensive — decline; with strings — decline and name the string. (5) Notice; ask what it means; decide on the category; say the decision once, warmly; record it. (6) Client initiates, value agreed in writing, time-limited, supervised and recorded; the hazard is that the client's service becomes clinical material and a second arena in which she can fail. (7) Package discounts; productivity targets; upcoding; referral payments; testimonials and advertising claims; products sold to clients; the consultancy overlap; unbilled work. (8) Accept it and ask the

family to sell it and put the money towards the fee — preserving the meaning of the gift, avoiding an asset transfer, and recognising the family's own form of payment.

12.12a Three sentences to keep in the drawer

"Tell me early." Said at intake and repeated at every review, it is the sentence that prevents the silent ending.

"What would it mean to you if I accepted this?" The gift question that converts an object into a conversation.

"We will write it down and look at it again on [date]." The sentence that turns generosity into a professional arrangement, and the one that protects both parties from the drift into gratitude and resentment.

12.13 Reflective task

Write two hundred and fifty words on your own money history: what you were taught about asking to be paid, what you were taught about accepting gifts, and which of those lessons is most likely to shape your practice. Then name one rule you will write down before you see your first paying client.

12.14 Further reading

- Any current text on the business of private practice in your jurisdiction — read for the arithmetic, not the marketing.
- Pope, K. and Vasquez, M. on gifts, fees and barter in psychotherapy — the standard treatment of the ethical questions.
- The professional code sections on fees, gifts, barter and advertising, read in the original wording.
- The literature on conflicts of interest in medicine — the evidence that financial interest shapes judgement while clinicians believe it does not.
- Your indemnity provider's guidance on fees, debt recovery and court attendance charges.

Next: Week 13 — Ethics of research, assessment and diagnosis.

Week 13 — Ethics of research, assessment and diagnosis

Learning outcomes. By the end of this week the student can (1) obtain valid consent for research with families, including assent from children and the problem of group consent; (2) write about a client without identifying her, and state the limits of disguise; (3) describe the consequences of a diagnosis and decide what she can defend; (4) select and interpret an assessment instrument with attention to its validity for the population in front of her; and (5) explain the ethical difference between the clinician-researcher and the researcher who happens to be a clinician.

Three domains, one theme

Research, assessment and diagnosis are usually taught in three separate weeks and they share a single ethical structure: **each of them gives the therapist power to define a person.** The researcher defines what will be known about a family and who will read it. The assessor defines what will be counted as a problem and how it will be scored. The diagnostician defines what the person is, in a category that will follow her into an insurance form, a school file, a custody hearing and her own understanding of herself.

That power is not a reason to avoid the three activities; a profession that does not research, assess and diagnose is not a profession. It is the reason each of them requires a specific discipline.

Domain	The duty	The characteristic failure
Research	Voluntary, informed, revocable consent; protection of the vulnerable; honest reporting	The family "agrees" because the therapist asked; children's assent treated as unnecessary; the negative result not published
Assessment	Competence to administer; validity for the population; feedback to the client	An instrument normed on another population, interpreted as though it measured the same thing
Diagnosis	Defensible, useful, communicated honestly; benefits weighed against stigma	A diagnosis given for the insurer, the school or the court rather than for the client

13.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The single theme: power to define	Three examples, students identify who was defined
8–22	Five concepts: consent in families, the diagnostic act, cultural validity, the assessed client, the writing therapist	Lecture
22–35	Consent with families: the father problem, the child's assent, withdrawal by one member	Two ethics applications, compared
35–50	Writing about clients: consent, disguise, the small-community problem	Students disguise a real case in twenty minutes; group tries to identify it
50–62	Diagnosis: consequences and defensibility	Four diagnoses, students argue each is or is not defensible
62–75	Instruments: validity, qualification, feedback	Three instruments presented; students ask the five questions
75–85	The clinician-researcher: recruitment, dual role, authorship	Case of a thesis that went wrong
85–90	The Amaka case	Students decide whether the study proceeds

13.2 Five concepts that organise the week

Concept 1 — Consent in a family is not one person's signature. A father can consent to his own participation and cannot consent to his wife's or his children's. A child's assent is required in addition to a parent's consent where the child is old enough to understand, and the child's refusal should be respected even where the law does not require it. A mother who consents under pressure from her husband has not consented. The research consent process in family work is therefore slower, more documented and more individual than in individual research.

Concept 2 — Diagnosis is an act with consequences, and the consequences are distributed. The same diagnosis may give one person access to treatment, mark another as unsuitable for a job, settle a custody question, and change how a family talks about a member for the rest of his life. Diagnosing is not wrong; diagnosing without weighing the consequences is.

Concept 3 — An instrument measures what it was normed to measure, in the population it was normed on. A questionnaire developed and validated on one population, translated and used on another, may measure something different or nothing at all. The ethical problem is not the use of a tool; it is acting on a score whose meaning in this population is unknown.

Concept 4 — The assessment has a client, and it is not always the person being assessed. In family work the assessor must state whose question the assessment answers. A child assessed at a school's request, a couple assessed for a court, a father assessed for an insurer: each has a different client, a different consent requirement and a different limit on what can be reported back.

Concept 5 — The therapist who writes is still a therapist. Case reports, theses and teaching material are written by people who know their subjects personally. The obligations are consent, disguise, proportionality (writing only what the purpose requires), and — in small communities — a willingness not to write at all.

13.2a Research consent with families: six problems and their solutions

Problem 1 — the father signs for everyone. Solution: separate consent forms for each adult; an assent form for each child old enough to read one; a note of who was present when each was signed; and no recruitment in a session where one member is dominant.

Problem 2 — the therapist recruits her own clients. Solution: recruitment by a third party who is not the treating clinician, or a clear statement in the invitation that declining will not affect treatment, made by someone other than the therapist. Where the therapist must recruit, the invitation is written, the client is given a week, and the therapist does not ask again.

Problem 3 — one member wants to withdraw. Solution: a stated policy at the outset. In most family studies the unit is the family, and the withdrawal of one member means the family's data are withdrawn; where the design permits partial participation, that is explained in advance. The member who withdraws must not be told by the researcher that they have "ruined" the study.

Problem 4 — children's assent. Solution: an age-appropriate explanation, a written or verbal assent, and the child's right to stop at any point without anyone being told why. Children are frequently the most reliable participants on this question and the least often asked.

Problem 5 — confidentiality inside the research team. Solution: stated at consent: who will see the data, whether transcripts will be shared, how identifiers will be removed, and where data will be stored. A family consenting to research has not consented to being discussed by a research team in a seminar.

Problem 6 — the participant in crisis. Solution: a written procedure for what happens if a participant discloses risk during data collection: the interview stops, the participant is supported, the clinical pathway is followed, and the researcher's duty is stated in the consent form. A researcher who is also a therapist must know which role she is in at the moment of disclosure, and the participant should have been told in advance.

The document that results. A consent form of two pages for adults, one page for children, an information sheet in plain language, and a procedure for the crisis. Four documents, and none of them is optional.

13.2b Writing about clients: consent, disguise and the limits of disguise

Consent to publish. Where a case is written up, the client's consent is obtained in writing, and it is consent to the specific text, not to "use my case". The client reads the text, or hears it read, and may require changes. Where the client is a child, the parent's consent and the child's assent; where the client has died, the family's consent and additional care, because the dead cannot be embarrassed and the living can.

Disguise, and the four things it cannot protect. Changing the name, the occupation, the town and the ages is standard and insufficient in three situations. Small communities: a therapist in a town of thirty thousand is writing about someone her colleagues will recognise from the presenting problem. Rare presentations: a family with a specific configuration — four children, a father in the merchant navy, a mother with a particular diagnosis — is identifiable no matter how the details are changed. Public events: a case that was in the newspapers is not disguisable. Digital traces: a case that has been discussed online may be found by searching the details.

The proportionality rule. Write only what the teaching purpose requires. A case report does not need the family's immigration status, their religion, their income or the detail of the abuse in order to teach the decision. The most common ethical failure in student case reports is excess detail, which is usually written because the detail is interesting rather than because it is necessary.

The composite case. Where disguise is impossible, a composite — several cases combined into one, with a statement that it is a composite — is acceptable in teaching and unacceptable where the composite is presented as a single case with its own factual integrity. The statement must be explicit: "this case is a composite, constructed for teaching, and does not describe any single family."

The student's thesis, and the supervisor's role. The supervisor is responsible for the ethics application and for reading the case material before submission. A thesis containing a recognisable account of a real family is a failure of supervision before it is a failure of the student.

13.2c The assessed client: whose question does the assessment answer?

An assessment is always conducted for someone. Naming that person determines the consent, the feedback and the limits, and the failure to name them is the commonest ethical error in assessment work.

The four possible clients of an assessment.

Who asked	Consent required	What may be fed back
The person being assessed	Her own	Everything, in a form she can use
The family, about one member	The member's, plus the family's for the referral question	The formulation, not the raw material
A school or employer	The person's written consent, and only for the agreed purpose	What the purpose requires, and nothing more
A court	The person's consent where possible, or the court's order	The report as instructed, with its limits stated

The distortion that follows from not naming the client. The therapist drifts towards the interests of whoever commissioned the work. A child assessed because the school wants a diagnosis is assessed in a way that finds one; a father assessed because his wife's lawyer requested it is assessed in a way that finds what the lawyer is looking for, or — in the therapist's attempt to be fair — in a way that under-reports what is there. Naming the client is the discipline that prevents the drift, and stating it in the report is what allows a reader to judge the work.

The feedback conversation, which is part of the assessment. Three questions: what was measured, what the result means, and what it does not mean. The client who is told a score without the third element will carry a number as though it were a verdict. "Your score is in the range that suggests moderate depression. That tells us the severity is significant enough to treat. It does not tell us the cause, and it does not tell us how long it will last."

The assessment that should not be done. Where the question belongs to a third party and answering it would harm the person assessed — a family who wants an assessment to prove that a daughter is "not really ill", an employer who wants an assessment of an employee's loyalty — the therapist's answer is that the question is not one she can answer, and the reason is stated.

13.3 Diagnosis: what it does, and when it is defensible

What a diagnosis does, in four directions.

1. **To the client:** it names the experience, which can relieve ("there is a name for this"), and it can imprison ("I am a borderline"). The same act produces both, and the way it is communicated determines which.
2. **To the family:** it reorganises the household's explanation. A diagnosis of a child's condition may relieve the mother of blame or transfer it to her; a diagnosis of a father's depression may legitimise his withdrawal or confirm the family's view that he is weak.
3. **To the system:** it opens the door to treatment, insurance, school support and legal protection — and it closes others. A diagnosis on a record follows a person into employment, custody and immigration decisions.
4. **To the therapist:** it organises the treatment and, if it is wrong, organises it in the wrong direction for years. The diagnostic act is a commitment, not a label.

The four questions before making a diagnosis.

1. **Can I defend it?** Is there enough evidence, gathered with the right method, to support it if challenged by a colleague, a regulator or a court?
2. **Is it mine to make?** Does it lie within my competence and my licence? A family therapist may recognise depression and may not be qualified to diagnose a psychotic disorder (Week 7).
3. **What will it do for this client?** Access to treatment, an explanation, a school's support — or stigma, a record, a family's certainty that the problem is located in one person?
4. **What will it do to this family's story?** Diagnosing the identified patient in a family system confirms the family's belief that the problem is in that person, which is frequently the belief that has been preventing change.

The relational alternative, and its limit. In family work the formulation is frequently relational rather than individual: "this family's difficulty is in how it has organised itself around a loss", rather than "this mother is depressed". The relational formulation is often more useful and is not a substitute for a diagnosis where a diagnosis is required for care. The competent practice uses both: a diagnosis where the system requires one, and a formulation that locates the problem in the relationships where it actually lives.

The diagnosis given for a third party. "I need a diagnosis for the insurance." "The school will not act without a diagnosis." "My lawyer says a diagnosis will help in court." Each is a pressure to write a category the client may not meet, for a purpose that is not clinical. The defensible position: a diagnosis is given if it is accurate; where a letter is needed for a purpose, the letter states what is observed and what is needed, without inventing a category. Where the third party will not accept that, the therapist declines and says why.

Communication of a diagnosis, in four sentences. What it is, in plain language; what it is not; what it means for the treatment; and what it does not change about the person. "This is a name for a pattern of low mood that has lasted more than two months and is affecting your sleep, your appetite and your work. It is not a description of who you are,

and it is not permanent. It tells me what treatment is likely to help. It does not change anything about what you are able to do."

13.3a Assessment instruments: six questions before you use one

1. Was it validated for this population? Not translated — validated. A translation with no psychometric work is a new instrument with unknown properties, and reporting its scores as though they meant what they mean in the original population is a methodological and an ethical error.

2. Was it normed on people like this client? Age, education, language, urban or rural, clinical or community sample. A cut-off derived from a clinical sample will classify a community sample as abnormal.

3. Am I qualified to administer and interpret it? Some instruments require specific training; all require competence in interpretation. Administering an instrument one cannot interpret is worse than not using one.

4. What will I do with the result? If the answer is "nothing that I would not do otherwise", the assessment is being done for the therapist's reassurance or for a file. If the answer is "it will change the treatment", the result must be fed back to the client.

5. How will I give the result back? The client is entitled to know what was measured, what the score means, what its limits are, and what happens next. Feedback is part of the assessment, not a courtesy.

6. Where does the raw data go, and for how long? Test data are clinical records: stored, retained and disclosed under the same rules (Week 11). Protocols are not kept in a drawer indefinitely, and they are not shown to anyone outside the treatment without consent.

The family-specific problem. Many instruments are designed for individuals and used on families by summing scores, which produces a number that means nothing. Family assessment requires either an instrument designed for family-level constructs (cohesion, adaptability, communication) or an explicit statement that the scores describe individuals within the family and not the family as a unit.

The one instrument most family therapists do use. The outcome measure, completed at each session or at intervals, which is also an intervention: it gives the client a voice in the treatment and gives the therapist feedback on deterioration. Its ethical requirements are the same as any other — validity for the population, explanation of the purpose, and a stated response when the score indicates deterioration.

13.3b The diagnosis of a child, and the family that receives it

The most consequential diagnostic act in family work, because it lands on a person who did not ask for it, in a system that will reorganise around it.

The four questions, applied to a child. Can I defend it? With a child, the evidence must come from more than one setting — home and school — and from more than one informant, because a child's behaviour in a clinic is a sample of one context. Is it mine to

make? Many child diagnoses require specialist assessment; the family therapist's task is recognition and referral (Week 7). What will it do for the child? Access to support in school, a treatment pathway, an explanation for the parents — or a label that follows him through school records. What will it do to the family's story? A diagnosis of a child's condition can relieve the parents of blame, which is sometimes a kindness and sometimes a way of not examining a household in which the child is frightened.

The conversation with the parents. Five elements: what the assessment found; what it did not find; what the diagnosis means and does not mean; what the parents can do that will help; and who else will be told, and what they will be told. The fifth element is the one most often omitted and the one the parents most need, because the diagnosis will travel — to the school, the church, the extended family — and the parents will be the ones managing its journey.

The conversation with the child. Age-appropriate, brief, and about function rather than category. A nine-year-old does not need the word; he needs to know that "your brain works in a way that makes sitting still hard, and there are things that help, and it is not because you are bad." A fifteen-year-old may want the word, the explanation of what it means, and the assurance that it is not the whole of him.

The label that becomes the family's explanation. "He is like that because of his condition" is a sentence that relieves and closes at the same time. The therapist's work, over months, is to hold both truths: the condition is real and the household's patterns still matter. That is the family therapist's particular contribution to a diagnostic case, and it is the reason a diagnosis alone is never a formulation.

13.3c The diagnosis as a family event

When a diagnosis enters a household it does three things at once, and the therapist should anticipate all three.

It assigns a location to the problem. The difficulty now lives in one person. That is frequently the family's preference and frequently the reason change is difficult: if the problem is in her, the rest of the household has nothing to change. The family therapist's contribution is to accept the diagnosis and to keep the household in the formulation: "the depression is real and it is in her. And a household in which nobody speaks before noon is part of what is keeping it going."

It redistributes responsibility. A diagnosis can relieve a mother of blame ("it is an illness, not my parenting") and can transfer it to her ("it is genetic, from your side"). Both movements happen in the same session and both are heard by the person diagnosed.

It changes the family's language. Members begin to use the diagnosis as an explanation, a weapon and a prediction: "you cannot help it", "it is your condition", "you will always be like this". The therapist listens for the third use, because it is the one that forecloses change, and names it: "when you say she will always be like this, what does that do to her?"

What the therapist does with all three. Assess the diagnosis' accuracy; communicate it in the four-sentence form; then run one session, early, on what the family now believes

about it. That session is frequently worth more than six months of work on the symptoms, because it determines whether the household will support the treatment or organise itself around the label.

13.4 The clinician-researcher: the thesis, the client and the dual role

The MFT 770 thesis requirement means that every student on this programme will conduct research, frequently while seeing clients, and very often on material that arises from their own practice. The dual role is the week's most practical problem.

The four ways the dual role distorts research.

Recruitment bias: the clients who agree are the ones who like the therapist, and they know the therapist's preferences. Consent under influence: the client who is asked by the person she depends on cannot truly refuse. Interpretation bias: the therapist-researcher sees her own clinical decisions confirmed by the data. Role confusion at the moment of disclosure: the participant says something that activates a duty, and the researcher must decide which hat she is wearing, in the middle of an interview.

The four controls.

1. **Recruitment by a third party** wherever possible, and where not, a written invitation delivered by someone else and a stated cooling-off period.
2. **A written statement of what the researcher will do if the participant discloses risk**, agreed with the ethics committee and stated in the information sheet.
3. **Analysis by someone other than the interviewer,** or at least a second coder who has no clinical relationship with the participants.
4. **Supervision of the dual role,** in which the student discusses the specific ethical events of the study as they happen, and a supervisor who reads the transcripts.

The ethics application, in five sections. Purpose, participants, consent, data, risks. The section that is most often thin is risks — not risks to participants' bodies, which are usually minimal in this kind of research, but risks to their relationships and reputations: a couple who both participate may disagree about what has been said; a transcript stored on a laptop may be read by a partner; a quotation may be recognised by the family. Each of these is a risk the application must name and mitigate.

Authorship and honesty. The student's thesis is the student's; the supervisor's contribution is acknowledged according to the convention of the institution. Data are not fabricated, not selectively reported, and not discarded because they do not support the hypothesis. The negative finding is a finding and is reported. A student who has learned to write up an unexpected result honestly has learned the whole of research ethics.

13.4a The seven-step research ethics check

A compressed procedure for the student designing the MFT 770 study, to be run before writing the application.

1. **Whose question is it?** The researcher's, the supervisor's, or a community's. If it is the researcher's alone, the study should say what it gives back.

2. **Who is vulnerable here, and in what way?** Not "vulnerable populations" in the abstract, but the specific people in this sample: the woman whose husband controls the household, the child who is asked about his parents, the couple in acute crisis.
3. **How will consent actually be obtained, in what language, by whom?** If the answer involves the treating therapist, the design has a problem.
4. **What are the relational risks?** What could be said in an interview that would be repeated at home, and what would happen then?
5. **What is the plan if a participant discloses risk?** Written, with names and numbers, and rehearsed.
6. **What will be published, and who could be identified by it?** Including the third parties who never consented.
7. **What does this community get?** A returned report, a training session, a copy of the thesis in the local library, a co-author from the setting. If the answer is "nothing", the study is extractive and the ethics committee may still approve it, which does not make it right.

The two questions that predict trouble. Question 3, because the dual role lives there, and question 6, because it is the one the student has not thought about and the one the supervisor will be asked about later.

13.5 Case: Amaka's study, and the participant who was recognisable

The student. Amaka, a candidate on this programme, is conducting her MFT 770 study on "the experience of forgiveness in couples after infidelity". She has recruited eight couples: three from her own caseload, five through a church announcement.

The design. Semi-structured interviews of ninety minutes with each partner separately, audio recorded, verbatim transcripts, thematic analysis, with the church's permission to announce the study.

The three problems that emerged.

Problem one, at recruitment. One of her own clients, Mr Osagie, offered to participate "because you have done so much for us". Amaka's first instinct was to accept, and the ethics application had specified that clients in current treatment would not be recruited. She declined, and the client experienced it as rejection; the next session was spent on that, which was the right clinical decision and a cost of the design.

Problem two, at interview. During Mrs Osagie's interview (she participated through the church route, not through Amaka), she described her husband's affair in a form of words identical to the account Amaka had heard in the couple session, and added a detail — the other woman's workplace — that Amaka already knew from the clinical file. Amaka stopped the recording and said, "I want to remind you that this is research, and that what you say here goes into the study and not into our sessions. Also, I already know some of this from our work, and I need you to know that I am not going to use anything in the study that I did not hear here, today, in this interview." She recorded that statement in the note and in the study log.

Problem three, at analysis. In the draft chapter, a quotation read: "She worked in the same bank as him. I used to see her at the Christmas party." With the ages, the city and the church, the couple would be identifiable to anyone in their congregation who read the thesis, and the thesis would be in the university library.

What Amaka did.

1. She removed the detail and replaced it with "she worked at his office", which preserved the meaning (proximity) and removed the identifier.
2. She changed the town in every extract, not only in the demographic table.
3. She wrote to both partners with the three quotations she intended to use, and asked whether they were content. Mr Osagie asked for one to be removed. It was removed, and Amaka noted the change.
4. She added a paragraph to the methodology describing the measures taken and their limits: "no disguise can guarantee anonymity in a small community; the participants were told this, and the extracts used were approved by them."

The supervisor's question, which produced the case. "If the other woman read this thesis, what would she know?" Amaka had not considered her — the third party, who has not consented to anything.

What the case teaches.

1. **The dual role has a specific failure point,** which is recruitment. The application's prohibition was right and its cost — a difficult session — was the price of doing it properly.
2. **The moment of overlap between the clinical and the research roles must be handled out loud.** Amaka's statement in the interview was not required by any code; it was the honest management of a boundary that the participant could not have known was there.
3. **Consent to be studied is not consent to be recognised.** Sending the quotations to the participants is unusual and is the highest standard available.
4. **The third party is the person everyone forgets.** The affair partner, the child, the grandparent in the transcript: none of them consented to appear in a thesis, and all of them may be identifiable.

13.6 Transcript: the consent conversation for a case report

A therapist asks a former client for permission to write up the treatment for a teaching chapter. Minutes eight to twenty-two.

> **T1 Therapist:** I am writing a chapter for students about the kind of work we did, and I would like to use your case. (pause) Before you answer, I am going to tell you exactly what that would involve, and I want you to know that you can say no and it will change nothing between us. — The request, followed immediately by the two things that make it consent rather than pressure: the information and the right of refusal. **T2 Client:** What would you write? **T3 Therapist:** The decisions I made and why, and something of what you were dealing with, so that students can learn from it. Not your name, not your town, not your job as it is, not the names of anyone in your family. — The scope, stated positively and negatively, before the document is shown. **T4 Client:** But you would show it to people. **T5 Therapist:** It would be published, yes. (pause) That is the part I want you to think about properly, not answer now. (pause) Anyone could read it, including people who know you. — The risk named plainly, including the one the therapist would rather not mention: the possibility of recognition. **T6 Client:** Would they know it was me? **T7 Therapist:** They might. (pause) I will change the details, and I cannot promise that. Your church, your children's ages, the thing that brought you in — those together are quite specific, and I would rather tell you that than pretend the changes are a guarantee. — The honest answer to the question that decides the case. A therapist who says "no one will know" has obtained a consent that is not informed. **T8 Client:** (pause) If I said no, would you be upset? **T9 Therapist:** I would be disappointed and I would not be upset with you, and I would not mention it again. (pause) That is the difference between asking and pressuring. — The distinction stated because the client cannot know it otherwise, and because the therapist's disappointment is precisely what she must not transmit. **T10 Client:** Can I read it first? **T11 Therapist:** Yes. I will send you the text before it goes anywhere, and you can strike anything you want, and I will tell the editor that you have seen it. — The offer that converts the consent from consent-to-use into consent-to-publish-this-text, which is the standard the codes describe and which few therapists actually meet. **T12 Client:** (pause) And after — could I say later that I have changed my mind? **T13 Therapist:** Before it is published, yes. After, no — it will be out. And if you asked me before publication to stop, I would stop. — The limits of revocability, stated precisely. "I would stop" is a promise the therapist can keep and the client can rely on. **T14 Client:** (long pause) All right. Send it to me.

What the transcript shows. Six of the seven therapist turns give the client something that makes refusal possible: the scope, the risk, the honest answer about recognition, the assurance about disappointment, the right to read the text, and the right to withdraw before publication. Consent obtained without those six things is a signature, not a consent.

What happened afterwards. The client struck one sentence — a detail about her mother — and asked that the town be changed to a different region. Both were done. The published chapter carries a footnote: "details have been changed and the client has read and approved this account."

13.7 Laboratory: the ethics application and the disguised case

Part A — the application (forty minutes). In groups, write the five sections of an ethics application for this study: "the experience of disclosure of HIV status within couples, recruited through a clinic in [state]". Each group must include: the consent documents; the recruitment route; the plan for a disclosure of risk during an interview; the data storage and destruction plan; and the specific relational risks to participants.

Part B — the disguise exercise (twenty-five minutes). Each student writes 250 words about a case, disguised, and reads it aloud. The group then tries to identify the family from the details given. The exercise is uncomfortable and instructive: almost every first draft is identifiable.

Marks (20).

Criterion	0–1	2–3	4–5
Consent	One form	Separate forms	Separate forms for adults and children, with assent and withdrawal stated
Risk plan	None	Risk mentioned	Disclosure procedure specific, with roles named
Disguise	Not attempted	Names changed	Multiple details changed, third parties protected, limits acknowledged
Data	Not addressed	Storage mentioned	Storage, access, retention and destruction specified
Honesty	Overclaims anonymity	Some caution	Anonymous where possible, and the limits stated to participants

The observation for the debrief. No group's first draft will protect the third party — the affair partner, the grandmother, the sibling who appears in one sentence. That omission is the lesson.

13.7a Four research designs and the ethics of each

The design determines which ethical problems will arise, and students choosing a design are also choosing their ethical work.

1. The interview study. Risk: the disclosure of distressing material with no clinical container; the repetition of harm in a research interview with a survivor. Mitigation: the interviewer's training, a stated procedure if distress occurs, a debrief, and the contact details of a service.

2. The survey. Risk: consent that is a click; items that are intrusive in a household where the questionnaire may be seen; the impossibility of withdrawal once data are pooled. Mitigation: plain language, the option to skip items, a statement about who may see the completed form, and a clear withdrawal window.

3. The outcome study using clinical data. Risk: using records for a purpose the client did not consent to; anonymisation that fails in a small clinic; the therapist's interest in a positive result. Mitigation: a specific consent to the use of data, an independent analyst, a pre-specified analysis, and a commitment to report the negative result.

4. The action research or service evaluation. Risk: the conflation of evaluation with research; staff or clients who cannot refuse because the evaluator is their manager or therapist. Mitigation: independent facilitation, anonymised data collection, and a written agreement about what will be reported to whom.

The design choice in a small programme. Most MFT 770 studies will be interview or survey designs with small samples. The ethical burden in a small sample is not statistical; it is the impossibility of true anonymity and the closeness of the researcher to the participants. Both are manageable with honesty, and neither is manageable with a standard consent form.

13.8 Six errors

Error 1 — Recruiting your own clients because they are available. The consent is contaminated, the sample is biased, and the client's refusal becomes a clinical event. The prohibition exists for a reason and the reason is not administrative.

Error 2 — Treating a parent's signature as family consent. Each adult consents for herself; each child assents for himself; and the consent is documented per person.

Error 3 — Promising anonymity that the design cannot deliver. A promise of anonymity to a family in a small community, from a study that will be in a university library, is a false statement made in writing.

Error 4 — Diagnosing what the payer will reimburse. It is fraud, and it is also a clinical decision made by a third party who will never meet the client.

Error 5 — Using an instrument because it is available. Validity for the population is not optional, and acting on a score whose meaning is unknown is acting on nothing.

Error 6 — Writing about clients without telling them, because the writing is "for teaching". Teaching is a purpose, not an exemption; the consent is required, and the requirement is stated in most codes in terms that students frequently have not read.

13.9 Culture, faith and the ethics of knowledge

Who researches whom. A substantial literature on research in African settings documents the problem of extractive research: studies designed elsewhere, conducted on local populations, published in journals the participants will never read, with no benefit to the community studied. The ethical response is not to stop researching; it is to build the study so that the community gains something — findings returned to the clinic or the church that hosted the study, local co-researchers, a dissemination event in the participants' language.

Community consent. In settings where a community leader, a church or a family elder is the gatekeeper, formal consent processes frequently require the gatekeeper's permission in addition to individual consent and never instead of it. A study in which the pastor agrees and the women do not is a study conducted on people who did not agree.

Instruments from elsewhere. Most widely used instruments were developed in North America or Europe. Their use in this region raises the questions of translation, conceptual

equivalence (does the construct exist in the same form in this language?), and norming. A depression scale that asks about "feeling sad" may miss a presentation organised around bodily complaints, which is a well-documented pattern in many settings. The ethical requirement is honesty about the limits, not the abandonment of measurement.

Diagnosis across cultures. The cultural formulation in DSM-5 and the ICD guidance on cultural concepts of distress exist because the same experience is described differently across settings. The practitioner's discipline: describe the experience in the client's terms first, then consider whether a category fits, and record both.

Faith and the diagnosis. A family may hold that the problem is spiritual, and a diagnosis may be heard as a denial of that. The practitioner's position — Week 7's — is additive rather than competitive: the diagnosis and the family's explanation can coexist, and the therapist's task is to work with both without requiring the family to give up its account.

> **DEBATE**
> "Research on families should require the consent of every adult member, and any member's refusal should end the study for that family." Argue for: the family is a unit and the data are relational, one member's refusal will distort what the others say, and the family's privacy is shared. Argue against: an individual's participation is her own decision, a controlling husband could veto his wife's participation, and the rule would silence precisely the people whose voices are least often heard. Then state the rule you would put in your own ethics application.

13.9a Writing the thesis: five sentences the examiner will look for

Not a style guide; a list of the sentences that demonstrate the student has understood the ethics of her own study.

1. **"Participants were recruited by [a third party / letter] and were told that declining would not affect their care."** The sentence that shows the dual role was considered.
2. **"Each adult gave written consent; each child gave written or verbal assent, recorded in the file."** The sentence that shows consent was individual.
3. **"The sample was drawn from a single clinic and anonymity cannot be guaranteed; participants were told this."** The sentence that shows honesty about limits.
4. **"Where a participant disclosed risk during the interview, the procedure agreed with the ethics committee was followed: [what happened]."** The sentence that shows the protocol was not decorative.
5. **"The findings did not support the expected relationship between [x] and [y]."** The sentence that shows the student reported what she found.

The missing sixth, which distinguishes a good thesis from a competent one. A short paragraph on what the study gave back: the report returned to the clinic, the session run for the staff, the plain-language summary for the participants. Students who

write this paragraph have understood that research is a relationship and not a transaction.

13.10 Evidence note

What is known. The evidence on research ethics is largely in the form of guidance and documented failures rather than trials: consent processes that participants do not understand, therapeutic misconception (participants believing that research is treatment), and the systematic under-representation of children's own assent. In assessment, the cross-cultural validity literature consistently shows that instruments do not travel without psychometric work, and that translated tools can produce artefactual differences in prevalence. In diagnosis, research on labelling documents effects on self-concept, on family attributions and on clinicians' subsequent information-gathering — the diagnostic label shapes what is noticed next.

What is weaker. There is little empirical work on how therapists actually obtain consent to publish, and case reports are almost never audited for identifiability. The evidence on the harms of diagnostic labelling in family therapy specifically is thin.

What is not supported. That anonymity is achieved by changing names. That a diagnosis is harmless because it is confidential — the client's own family, school and employer frequently learn it. And that a validated instrument used outside its validation population measures the same construct.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same diagnosis given at the start of a treatment, as a working formulation, and given at the end, as a verdict on a person, has two different effects on a family.

13.11 Key terms

Assent — a child's agreement to participate, required in addition to a parent's consent.

Therapeutic misconception — the participant's belief that research activity is treatment.

Cultural validity — the extent to which an instrument measures the same construct in a different population.

Diagnostic act — the decision to assign a category, with its consequences for access, stigma and identity.

Composite case — a teaching case constructed from several cases, and labelled as such.

Extractive research — research that takes data from a community without returning benefit.

13.11a The five records a research-minded clinician keeps

A clinician who intends to write, teach or research will need material, and the material must be collected lawfully from the beginning.

1. **A consent-to-contact register,** separate from the clinical file, recording who has agreed to be approached about research or teaching and when. It is the single document that makes later recruitment lawful.
2. **A teaching-consent note** in the file, with the specific scope: "agreed to the use of anonymised material from this treatment for teaching and publication, subject to seeing the text; consent recorded [date]".
3. **A research log,** for any study: recruitment, withdrawals, the date of each consent, and every ethical event (a disclosure of risk, a request to withdraw, an adverse reaction) with the action taken.
4. **A data map,** one page: what data exist, where they are stored, who can access them, and when they will be destroyed.
5. **A publication file,** containing the approved text, the participant's written approval, and the correspondence with the journal. It is the evidence that the consent was to that text and not to a general idea.

Why the fifth record is the one students omit. Because consent is usually obtained at the beginning, when the text does not yet exist, and the version published may differ from the version described. The file closes that gap and takes ten minutes.

13.12 Self-test

1. Why is a father's signature not sufficient consent for a family study?
2. Give three things disguise cannot protect, and the rule that follows.
3. What are the four questions before making a diagnosis?
4. State the six questions before using an assessment instrument.
5. What are the four ways the clinician-researcher's dual role distorts research, and the four controls?
6. What is therapeutic misconception, and how is it prevented?
7. In the Amaka case, what did she do at the moment of overlap in the interview, and why?
8. What is a composite case, and when may it be used?

Answers. (1) Because consent is individual: the father consents for himself, the mother for herself, and each child assents; a mother consenting under pressure has not consented. (2) Small communities, rare presentations, public events (and digital traces); the rule is that consent is obtained to the specific text, the limits are stated, and where disguise cannot protect, the case is not published. (3) Can I defend it; is it mine to make; what will it do for the client; what will it do to the family's story. (4) Validated for this population; normed on similar people; am I qualified; what will I do with the result; how will I give it back; where do the data go. (5) Recruitment bias, consent under influence, interpretation bias, role confusion at disclosure; controls are third-party recruitment, a written risk procedure, independent analysis or second coding, and supervision of the dual role. (6) The participant's belief that research is treatment; prevented by explicit information, separate consent, and the researcher's refusal to blur the roles. (7) She stopped the recording and stated aloud that the interview was research, that nothing from

the clinical file would enter the study, and that she would use only what she heard in the interview. (8) A case constructed from several cases for teaching, labelled explicitly, and not presented as a single factual case.

13.12a Three questions the ethics committee will ask

Not the ones students prepare for.

"What will you do if the participant becomes distressed?" The answer must be a procedure, not a sentiment. Who stops the interview, what is said, who is called, and what the participant is offered afterwards.

"How will you know whether they understood the consent?" The answer is a method — a teach-back question, a second conversation, a written summary in the participant's language — not a signature.

"What happens to the data if you leave the programme?" The answer belongs to the institution, not the student: custody, storage and destruction specified in the application and followed at the end.

13.13 Reflective task

Write two hundred words about a family you have already met — in placement, in your own practice, or in this book's cases. Then answer: what would you have to change before you could publish it, and would what remained still teach anything? If the answer is no, you have found the reason that most good clinical writing is never written. If the answer is yes, write the limits into the text: what you changed, why, and what the reader should therefore not assume — because a reader who is told the limits can use the case, and a reader who is not will generalise it into a rule, and rules made from disguised cases have a way of being applied to real families who do not resemble the one described.

13.14 Further reading

- The Declaration of Helsinki and the CIOMS guidelines — short, and the source of most national codes.
- Your institution's research ethics guidance, read before writing the application rather than after the first rejection.
- The DSM-5 cultural formulation and the ICD guidance on cultural concepts of distress.
- Any current text on cross-cultural assessment and test adaptation — the psychometric requirements, not only the translations.
- The literature on research ethics in African settings, particularly on community consent and benefit sharing.

Next: Week 14 — Integration: the decision model and your stance.

Week 14 — Integration: the decision model and your stance

Learning outcomes. By the end of this week the student can (1) apply the seven-step decision model to an unfamiliar dilemma under time pressure and defend each step; (2) identify the three ways the model is misused; (3) write a stance paper that names what she will never do, what she will always consult about, and where her own values are most likely to distort her judgement; and (4) present a case to an examiner in a way that shows reasoning rather than recall.

What integration means at the end of fourteen weeks

The course began with the question of what kind of professional a person becomes when she takes on the codes, and it ends with the same question, now answerable. Everything between — consent, confidentiality, secrets, boundaries, competence, risk, law, records, money, research — is a set of particular decisions that a stance has to hold together. A therapist who knows all fourteen weeks and has no stance will make each decision freshly, under pressure, according to how she feels that afternoon. A therapist with a stance has already decided the large things, which is what leaves her free to attend to the client in front of her.

The course's claim, in one sentence. Ethical practice is not the avoidance of wrongdoing; it is the maintenance of a structure — consent, competence, supervision, records, consultation — that makes good decisions possible when the therapist is tired, frightened, fond of the client, or alone.

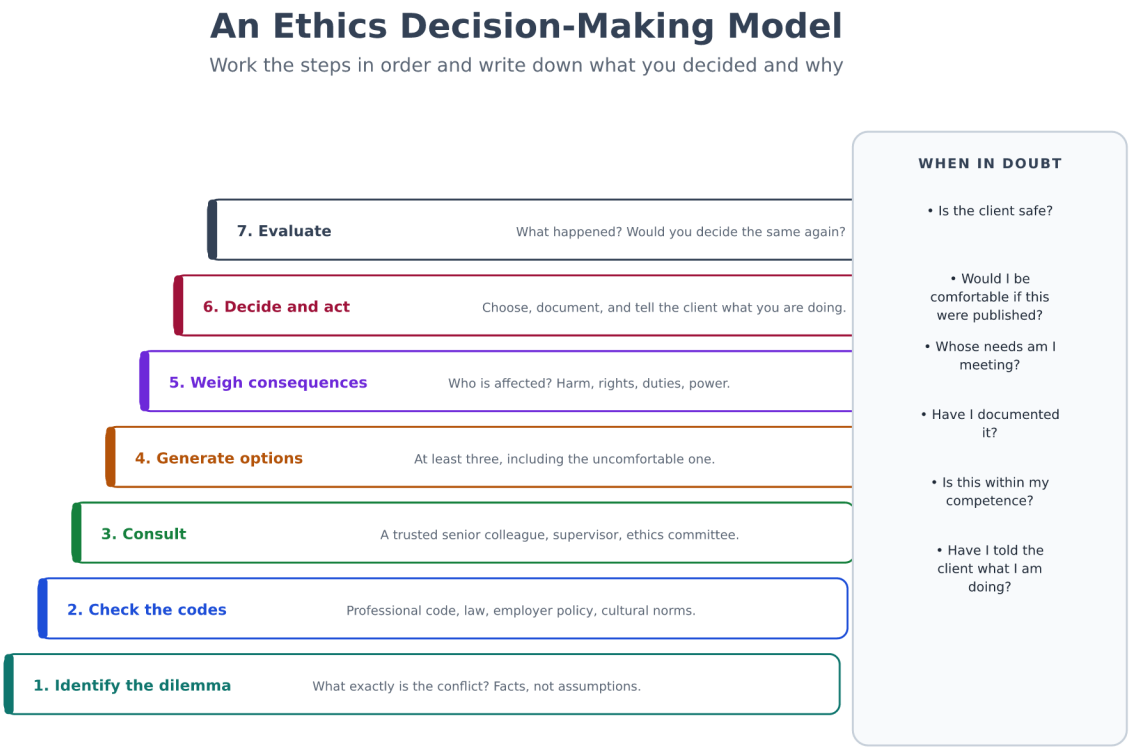


Figure 14.1 — A seven-step decision-making model for ethical dilemmas

14.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	What integration is; the stance and why it is the assessment	Two therapists, same dilemma, different stances
10–30	The seven steps, each with its failure mode	Lecture with a running case
30–45	The model applied end to end to a hard case	Whole-group application, one step at a time
45–55	The three misuses of the model	Three transcripts of reasoning, students identify the misuse
55–70	The stance paper: structure and worked extracts	Two anonymous stance papers, compared and marked
70–85	The viva: what examiners ask and what they are listening for	Demonstration viva with a volunteer
85–90	The closing question: what will you never do?	Written, collected, returned at the end of the programme

14.2 The seven steps, with the failure mode of each

Step 1 — Identify the dilemma. Not the situation: the dilemma. A situation is a mess; a dilemma is a choice between two goods or two harms. The statement to produce: "either I do X, and Y is lost, or I do Z, and W is lost." Failure mode: naming a situation as a dilemma when one option is obviously right — which is a way of avoiding the decision by making it look hard.

Step 2 — Check the codes and the law. Which clauses apply; which statute; what the jurisdiction requires. Failure mode: quoting a code from memory, or from another country. The check is documentary, and it takes ten minutes with the code in front of you.

Step 3 — Consult. A supervisor, a colleague, a lawyer, a specialist. Failure mode: consulting the colleague most likely to agree. The test of a consultation is that it changed something — a fact, an option, or the therapist's confidence in the wrong answer.

Step 4 — Generate at least three options, including the uncomfortable one. Three is the minimum because two options produce a binary in which one side is usually self-serving. The uncomfortable option is the one the therapist has not allowed herself to consider: ending the therapy, reporting, telling the client the truth, refusing the referral, transferring the case. Failure mode: the "three options" that are three versions of the preferred one.

Step 5 — Weigh the consequences for every affected party. The client, the partner, the children, the third party, the therapist, the profession, the future client who will be affected by the precedent. Written as a table, because a table exposes the party that has been left out. Failure mode: weighing only the client in the room, who is usually the most articulate and always the most present.

Step 6 — Decide, act, document. The decision is made, communicated to the client, and recorded with the reasoning, the consultation, the alternatives considered and the review date. Failure mode: documenting the decision without the reasoning, which is the note that reads as an assertion and cannot be defended.

Step 7 — Evaluate afterwards. What happened; what would be done differently; what the case has taught about the practice's structure. Failure mode: skipping step 7 because the case is over. This is the step that converts experience into competence, and it is the step that therapists in busy practices never reach.

14.2a The model applied, end to end

The case below is a composite, constructed to require every part of the course. It is the case used for the practical examination.

The situation. Ngozi A., 41, a nurse, has been in treatment for eleven weeks for depression following her husband's stroke. Her husband, Emeka, 52, is a retired teacher with significant disability from the stroke, cared for at home. Their daughter, Ada, 16, is in the household. In session eleven, Ngozi discloses that her brother-in-law has been sexually touching Ada for some months, that Ada told her in confidence, that Ngozi has not told anyone else, and that she has been "handling it by keeping him away from the house". She says she cannot manage her husband's care alone, that the brother-in-law is her husband's

younger brother and the family's only financial support, and that if this becomes public "they will say I destroyed the family." She is the therapist's neighbour's sister and the family attend the therapist's church.

Step 1 — the dilemma. "Either I report the disclosure, and this woman loses the support that keeps her household functioning and possibly her marriage, or I do not, and a sixteen-year-old remains at risk and the therapist becomes a party to the concealment." Stated as a dilemma, both losses are real. That is the point of step 1.

Step 2 — the codes and the law. The child protection duty is engaged: a child under eighteen has disclosed sexual abuse. The statute in the jurisdiction requires a report on reasonable suspicion, and here there is a disclosure. The code requires the therapist to report and to explain to the client what will be reported. The data protection framework permits the disclosure of the minimum necessary. The report does not require the mother's consent and does not require proof.

Step 3 — consultation. The supervisor, the child protection lead at the local authority, and — because of the dual relationship — the therapist's own supervisor's supervisor, or an ethics consultation. Read what happened in consulation: the child protection officer confirmed the threshold was met and gave the route and the name; the supervisor identified the dual relationship as a separate problem requiring a decision.

Step 4 — the options, including the uncomfortable one.

1. Report to the child protection unit today, tell Ngozi what will be reported, continue the treatment.
2. Report, and transfer the case because of the dual relationship and the threat to the alliance.
3. Support Ngozi to report herself, with a deadline of forty-eight hours, and report if she does not.
4. Do not report, and work with Ngozi to protect Ada within the family.
5. Report, and end the treatment.

Option 4 is unlawful and is stated in order to be rejected in writing, which is what a good record does. Options 1, 2, 3 and 5 are all lawful; they differ in what they protect. Option 3 is attractive and is wrong here: forty-eight hours of delay for a child in a household where the abuser has access.

Step 5 — consequences for every affected party.

Party	Option 1	Option 2	Option 5
Ada	Protection; investigation; possible family disruption	Same	Same
Ngozi	Told first; supported; may lose the therapist	Told; loses the therapist at the moment of crisis	Told; loses the therapist and the treatment
Emeka	Loses his brother's support; may learn of the abuse in a way he cannot control	Same	Same

Party	Option 1	Option 2	Option 5
The brother-in-law	Investigated	Same	Same
The therapist	Manages dual relationship and the alliance	Removes the dual relationship, abandons the client	Ends the case
The practice	A precedent for the dual-relationship question	Cleaner	—

Step 6 — the decision and the record. Report today; tell Ngozi in the session, in the words of Week 9; continue the treatment for the present, with the dual relationship managed and declared to the supervisor; review the decision to continue at the next supervision; record the seven steps in the file with the consultation names and times.

Step 7 — afterwards. At the review, three questions. Did the report protect Ada? Did the treatment survive? And the structural question: what in this practice allowed a therapist to be treating a family from her own church without a policy that would have raised the question at intake?

What the case demonstrates. The model does not make the decision comfortable, and it does not remove the losses. It makes the decision defensible, which is all a decision-making model can do. The therapist who reports is not choosing the child over the mother; she is choosing to comply with a duty that exists because adults in these situations frequently cannot protect the child themselves — including the mother, who is not a villain in this case and is not able to do what the law requires of the professional.

The teaching point students resist. The mother is not the problem. A model that turns her into the problem makes the case easier and the work worse. The step-5 table exists to keep her in the weighing.

14.2b The three misuses of the model

Misuse 1 — using it to justify a decision already made. The therapist reaches the conclusion in the first thirty seconds — usually the one that protects the relationship — and then walks the seven steps backwards. The signature is a step 4 that contains three versions of the same option, and a step 5 table in which the therapist's convenience does not appear because it has not been named.

Misuse 2 — skipping step 4. Two options, both bad, and the therapist chooses the less bad and calls it a decision. The uncomfortable third option is frequently the one that requires a conversation the therapist is avoiding — telling the client the truth, ending the treatment, refusing the fee, admitting the error.

Misuse 3 — treating the model as a substitute for a stance. A therapist with no position on the large questions will run the seven steps for every decision and produce technically defensible answers that do not add up to a practice. The model is a procedure for the difficult case; the stance is what handles the ordinary ones, which are most of them.

The test of a well-used model. After the decision, the therapist can state the option she rejected and why, the party she most regretted, and the part of the decision she found hardest. A therapist who reports no difficulty has probably not done step 5.

14.2c The six questions that find the dilemma

Before the seven steps, a filter. Six questions, used at intake and at every review, that identify the cases in which an ethical question is present before it announces itself.

1. **Who is not in this room, and would they be affected by what happens here?** The absent party is the commonest source of ethical error in family work: the affair partner's spouse, the child who is not brought, the husband who declined, the employer who is paying.
2. **What would I be uncomfortable reading aloud in a hearing?** The question that locates the weak point without requiring a theory of ethics.
3. **Am I the only person who knows this?** An isolated fact is a structural risk (Weeks 3 and 4).
4. **What have I been asked to do, by whom, in whose interest?** The referral question, the report, the character letter, the "just a quick word with the school".
5. **What am I avoiding asking?** The question the therapist has noticed and postponed. It is almost always the one that will become the dilemma.
6. **Is my practice built to catch this?** Policy, supervision, screening, records. If the answer is no, the dilemma is not a case problem; it is a practice problem that has produced a case.

Why the filter comes first. The seven steps are expensive in time and attention, and they are used on perhaps a dozen cases a year. The six questions take two minutes and are used on every case, which is where most of the protection lies.

14.3 The stance paper

The assessment for this week, and the document that outlasts the course.

What it is. Five hundred words in three sections, written in the first person, dated, and revisited at the end of every year of practice.

Section 1 — what I will never do. Six to ten statements, specific enough to be checkable. Not "I will always act ethically", but: "I will never see a client's partner without the client's knowledge"; "I will never accept a gift of money"; "I will never write an opinion about a parent I have not assessed"; "I will never continue a treatment in which I am the only person who knows something that would change it."

Section 2 — what I will always consult about. The list is shorter and it is the more important one, because the things on it are the ones the therapist will be tempted to handle alone. Typical entries: any case involving a child and a disclosure; any sexual boundary question; any decision to continue after a serious error; any case in which I notice that I am the only person the client trusts; any request I am inclined to grant as a favour.

Section 3 — where my own values are most likely to distort my judgement. The honest section, and the one that makes the other two credible. Examples of what a real entry looks like:

> I was raised in a household where a woman who left her husband was a failure, and I notice that I work harder with couples who are separating than with those who are not. I will consult whenever I find myself arguing for the marriage. I am uncomfortable with displays of anger, and I will notice when I am softening a client's anger on her behalf. I am frightened of being thought incompetent, and I will notice when a referral feels like a humiliation rather than a clinical decision. Money was never discussed in my family, and I will use the practice's policy rather than my own judgement at the moment the conversation becomes difficult.

Why the third section is examined. Because a therapist who cannot name her own distortions will not notice them in the room, and because the examined stance paper is the only place in the programme where the student is required to be specific about herself rather than about clients.

How it differs from a values statement. A values statement says what the therapist believes. A stance paper says what the therapist will do, refuse, consult about and watch for. The first is a position; the second is an instrument.

The revision. Once a year, in writing: which of the statements in section 1 did I break; which of the entries in section 3 was accurate; and what new entry belongs there. The revision is the record of professional development that no CPD log records.

14.3a The worked stance paper, in part

An extract from a student's paper, included because the specificity is instructive and because students consistently write something more abstract on the first attempt.

> **WHAT I WILL NEVER DO.**
> I will never treat a member of my own family, or a close friend, or the child of either, and I will say so at the point of referral rather than after the first session. I will never conduct a session without knowing whose client system I am working with. I will never let a couple's file contain a secret that either partner can demand to read. I will never give a diagnosis I have not written the reasoning for. I will never sign a letter I would not read aloud in court. **What I will always consult about.** Any disclosure of abuse. Any case in which I want to break a rule because the client's situation is exceptional — my exceptions are the least reliable part of my judgement. Any case in which I have begun to dislike the client. Any case in which I have become the only person the client talks to. Any fee arrangement I have made in the room rather than from the policy. **Where my values distort me.** I grew up in a home where conflict was forbidden, and I have discovered in role-play that I interrupt anger. I will ask my supervisor, every month, whether I have done it. I was raised in a church where divorce was a failure, and I have already noticed myself looking for reasons the marriage should survive in a case where the wife has not asked me to. My father was a violent man, and I am alert to the risk that I will see violence where there is only frustration, which is the opposite error and equally harmful. I am uncomfortable charging people who remind me of my mother, and I will use the sliding scale rather than decide in the room.

What makes this a good paper. Every statement is checkable, most are testable in a role-play, and the third section contains at least one entry that the student would rather not have written. That last feature is the marker of authenticity: a stance paper with no uncomfortable entry has been written for the examiner.

14.4 The viva: what is being examined

The form. A case presented by the candidate, usually one from her own practice or placement, followed by questions. Twenty to thirty minutes. The examiner has read the case summary; the candidate has the file.

What the examiner is listening for. Five things, in order of importance.

1. **Whether the candidate knows whose client it is.** The first question is almost always about the client system, and it is asked because candidates who cannot answer it are not safe to practise.
2. **Whether the candidate can name the ethical question without prompting.** The blank look followed by "there wasn't really an ethical issue" is the commonest failure, and it is a failure of recognition rather than of knowledge.
3. **Whether the reasoning is visible.** Steps taken, alternatives considered, consultation sought. The candidate who says "I decided to report" scores less than the one who says "I considered three options, and here is why I rejected two".
4. **Whether the candidate can distinguish what she knows from what she believes.** "I formed the impression" is acceptable; "I knew" is not, unless it was known.
5. **Whether the candidate's stance is present.** Not a speech about values — the ordinary consistency of a person who has decided the large things.

The case to bring. One in which the candidate made a decision she can defend, and one thing she would do differently. The case in which everything went well teaches the examiner nothing and the candidate less. The candidate who brings a case she got partly wrong, and can say what she learned, is marked higher than the candidate who brings a success.

The three questions that are asked in some form every time.

- "Who was your client?"
- "What was the ethical question, and when did you first notice it?"
- "What did you not do, and why?"

Preparing. Write a one-page summary: the family, the presenting problem, the decision point, the options, the consultation, the outcome, the reflection. Read the file. Know the dates. Rehearse the answers aloud to a colleague, because the first time these questions are answered should not be under examination.

The ethics of discussing cases in a viva. Consent from the client is not usually required for an examination conducted under confidentiality, and the candidate should nonetheless disguise the case and should say so: "the details have been changed." Examining boards treat the material as confidential; the candidate must treat it the same way outside the room.

14.4a Five answers that score badly, and their replacements

Poor answer	Why it fails	Replacement
"I felt it was the right thing to do."	No reasoning	"I weighed three options; the deciding factor was X; here is what I recorded."
"The code says I had to."	No judgement	"The code required a report; the judgement was in what I told the client and when."
"I did not want to damage the relationship."	The therapist's comfort as the criterion	"I considered the alliance, and I judged that the duty outweighed it; here is how I protected the relationship in the telling."
"It was a difficult case."	Says nothing	"It was difficult because two duties conflicted; the conflict was between X and Y."
"I would do the same again."	No learning	"I would do the same on the report, and differently on the timing — I would have raised it in session eight."

The pattern in all five. The poor answer describes a state; the good answer describes a decision. The examiner is not asking whether the candidate is a good person. She is asking whether the candidate can show her work.

14.4b The four sentences of the professional apology

Every therapist will make an error that harms a client. What happens next is a test of the whole course.

The four sentences. "I made a mistake and I want to tell you what it was." (Specific, owned, no passive voice.) "I am sorry, and I am not asking you to reassure me." (The apology without the request for absolution, which is what makes it usable.) "Here is what I have done about it." (The correction: the supervision, the referral, the record, the change.) "Here is what I will do next, and what you can decide." (The return of agency to the client, including the right to complain.)

The apology that fails. "I am sorry if you feel..." — an apology for the client's feeling. "Mistakes happen." — a generalisation that removes the therapist. "But you have to understand the position I was in." — an explanation that asks the client to carry the therapist. And an apology made so that the client will comfort the therapist, which is a request and not an apology.

What the codes require, and what they do not. The codes require honesty, the correction of the record where appropriate, and the client's right to complain. They do not require self-flagellation, and a therapist who collapses will make the client responsible for her.

The follow-up. The mistake is recorded, presented in supervision, and — where it reveals a structural problem — changes something in the practice. The client is told what changed. That last sentence is the one that converts an error into a demonstration of how a professional relationship handles failure.

14.5 The final supervision

The last supervision of the training year, in which the candidate presents her integration case. What should happen in it, and what the supervisor is looking for.

What the candidate brings. The case; the decision; the record of the decision; the stance paper; and one thing about her own practice that the training has changed.

What the supervisor asks.

- "What would you do differently if you met this family again tomorrow?"
- "Which part of your stance paper did this case test?"
- "What did you do that you did not tell me about at the time, and why?"
- "Who will you consult about this kind of case when I am no longer your supervisor?"

The last question is the one that matters. The purpose of supervision during training is not to make the candidate dependent on a supervisor; it is to build the habit of consultation into a professional life that will have no one appointed to provide it. A candidate who can answer the fourth question precisely — a named person, a peer group, a paid consultation, a specialist — is ready to qualify.

What the supervisor writes. A short assessment of the candidate's readiness that names the strengths and the specific development need going forward. Not a character reference, and not a summary of the case: a statement about what this clinician can be trusted with and what she should not yet do alone.

14.5a The case summary the candidate brings

One page, six headings, written the week before the viva. A worked example, drawn from a completed training case.

> **THE FAMILY.**
> The Oyelarans: Tunde, 46, a mechanic; Kemi, 43, a trader; children Segun, 15, and Bisi, 9. Referred by the school following Segun's attendance. **The presenting problem.** Segun's school refusal, which the parents attributed to "bad friends" and the school to "the home". The first session produced three different accounts and no agreement about the problem. **The decision point.** Session four: Kemi disclosed, alone, that Tunde had been drinking heavily since losing his workshop, that there had been two episodes in which he had pushed her, and that Segun had witnessed the second. She asked me not to tell him that she had told me, and not to raise it in the family sessions, because "it will make it worse". **The options I considered.** (1) Continue the family work and hold the disclosure, working individually with Kemi. (2) End the family work and see both adults individually. (3) Name the violence in the family session without attributing the disclosure. (4) Refer the family to a specialist domestic violence service and end the treatment. **What I did and why.** I did not continue the family sessions as though nothing had been said — option 1 would have made me a participant in the concealment, and Segun's witnessing made the children's exposure a child protection question. I did not confront Tunde with the disclosure (that would have exposed Kemi). I screened for lethality (no strangulation, no threats to kill, no weapon; two pushes; escalation with the drinking); I made a child protection referral regarding the children's exposure, told Kemi what I would say before I said it, and arranged individual work with her and a separate referral for Tunde to a substance use service. I did not end the treatment. I recorded the decision, the consultation (supervisor, [date]) and the review date. **What I would do differently.** I would have screened both partners individually in sessions one and two, as the practice protocol requires, rather than in session four when Kemi volunteered it. The protocol existed; I followed it late. And I would have asked Kemi directly whether she had told anyone else, because her sister had known for three months and was the person who had been keeping Segun overnight. **What the case taught me about my practice.** I avoid asking about violence in the first session because I am afraid of what I will then have to do. That is a stance problem, not a knowledge problem, and it is now in my stance paper.

Why this summary works. It states the dilemma without dramatising it, shows the rejected options with reasons, records the consultation, names the mistake without being destroyed by it, and connects the error to the therapist's own pattern rather than to the difficulty of the case.

14.6 Transcript: twelve minutes of viva

A candidate answering the three inevitable questions. Annotated.

> **Q1 Examiner:** Who was your client? **A1 Candidate:** The parents were the referrers and the school was the referrer of record. I treated the couple as the client system and I saw each of them individually under the policy I had stated in session one — which, as I will say, I did late. Segun was a client for three sessions, with his parents' consent and his own assent, and I was clear with all of them that what he told me would be shared with them only if he was at risk. — The answer names the system, the consent, the policy and the exception, in one turn. The examiner now knows most of what she needs. **Q2 Examiner:** When did you first notice there was an ethical question? **A2 Candidate:** In session four, when Kemi told me. (pause) And honestly, I should have noticed in session one, when the three accounts of the problem did not fit and nobody in the family would look at anybody else. — The honest answer with the retrospective recognition, which is what the examiner is testing: not whether the candidate noticed early, but whether she can tell the difference between noticing and understanding. **Q3 Examiner:** What was the question? **A3 Candidate:** Whether to keep the disclosure, which would have let the family work continue and would have made me part of the concealment, or to act on it, which meant the risk that Kemi would lose the only support she had. (pause) And underneath that, whether the children's exposure was a child protection matter, which it was. — Two duties named, and the second question identified as the one that decided the case. **Q4 Examiner:** Why did you not confront the husband? **A4 Candidate:** Because Kemi would have paid for it. (pause) The evidence is that conjoint confrontation in coercive situations escalates afterwards, and I had no way of predicting what he would do in the house that night. — The answer that shows the candidate has read the material and has applied it to a specific person's safety, rather than reciting a rule. **Q5 Examiner:** Did you consider referring the whole family out? **A5 Candidate:** I did, as option four. I rejected it because Kemi had disclosed to me and would not have disclosed to a stranger, and because a referral at that point would have been a way for me to avoid the decision rather than a service to her. (pause) I would refer now, if the violence escalated or if Tunde began to attend and I found myself holding both accounts. — The rejected option, the reason, and the condition under which the decision would change. This is the shape of a good answer. **Q6 Examiner:** What would you do differently? **A6 Candidate:** I would screen in sessions one and two rather than four. And I would not have assumed that because Kemi had not told me, nobody knew. Her sister knew. The sister had been keeping Segun overnight for three months, and I had not asked who else was involved. — A specific, small, real error, offered without self-punishment. **Q7 Examiner:** What does that tell you about your practice? **A7 Candidate:** That I delay asking about violence because I am afraid of the answer, and that the delay is itself a clinical decision with consequences. I have written it into my stance paper, and I have changed my intake so that the screening questions are asked in the first session of every couple. — The answer that connects an error to a change in structure, which is what makes a training case worth examining.

What the transcript shows. The candidate never defends, never performs regret, and never claims more than she knows. Her answers are short, specific, and each one contains a decision rather than a feeling. The examiner's follow-up questions get shorter as the viva proceeds, which is what happens when the answers are sufficient.

14.7 Laboratory: the mock viva and the stance review

Part A — the mock viva (sixty minutes). Students in threes: candidate, examiner, observer. Two rounds of twenty minutes, with ten minutes of feedback each. The candidate brings a one-page case summary; the examiner uses the seven questions from 14.4 and 14.6; the observer marks against the rubric below.

Criterion	0–1	2–3	4–5
The client system	Unclear	Named	Named, with consent and policy, and the exceptions stated
Recognition	Did not identify the question	Identified late	Identified, with an honest account of when and why
Reasoning	Conclusion only	Reasoning partly visible	Options, consultation, decision and record all visible
The mistake	Denied or catastrophic	Admitted	Named specifically, with the change it produced
Stance	Generic values	Some personal material	Specific, checkable, connected to this case

Part B — the stance review (thirty minutes). In pairs, students exchange stance papers and apply three tests. Is each statement in section 1 checkable? Convert any that are not. Is section 2 specific enough that a supervisor could hold the student to it? Does section 3 contain at least one entry the student would rather not have written? Papers that pass all three are read aloud to the group, and the group identifies which sentence would be hardest to keep.

The question to end on. "Which statement in your paper will be hardest to keep in your first year of practice, and what will make it hard?" The answers — money, a client's distress, the fear of losing the work, a supervisor who disagrees — are the actual curriculum of the first year.

14.8 Six errors of integration

Error 1 — Treating the model as a decision-maker. The seven steps produce a record of reasoning; they do not choose. The therapist who expects the model to make the decision will run it twice and still be undecided.

Error 2 — The stance paper as an essay about virtue. Written in the language of values, unverifiable, and forgotten by the second month. The paper is a set of commitments, and its test is whether it constrains behaviour.

Error 3 — Learning 700 pages and practising none of it. The student who can recite the codes and has not built a supervision arrangement, a records policy or a fee policy has learned the subject and not the practice.

Error 4 — The belief that a good therapist does not need structure. Structure is what allows the therapist to be fully present: the policy holds the decisions that would otherwise be made in the moment, under pressure, by a tired person who likes the client.

Error 5 — Recognising the ethical question only in retrospect. The commonest failure in the viva and in practice. The remedy is a checklist embedded in the ordinary workflow — screening questions in every first session, a consultation prompt in every complex case, a review date in every decision — because a habit fires where recognition does not.

Error 6 — Practising alone. Not a moral failure and a structural one: the therapist with no supervisor, no peer group and no consultation is the therapist in whom errors go uncorrected for years. The course's final recommendation is a relationship, not a rule.

14.8a The practice that is ready: a checklist of twelve

The course has argued throughout that ethical practice is structural. This is the structure, as a checklist that a graduate can work through in a fortnight and a supervisor can audit in an hour.

Documents. A consent form naming the limits and the client system; a confidentiality and secrets policy; a fee and cancellation policy; a records and retention policy; a disclosure protocol for requests from third parties; a complaints procedure; a list of verified referral contacts.

Relationships. A named supervisor with a written contract; a peer consultation group of at least three; an indemnity arrangement with access to legal advice; a professional body membership in good standing.

Habits. Screening questions in every first session of a couple or family; a supervision slot in the diary that is not moved; a note written within the day; a review date on every significant decision; one annual reading of the code and one annual audit of the practice's money.

The test. For each of the twelve, answer: does this exist in writing, and could a colleague find it without me? A practice in which the answer is yes for eight of the twelve is a practice that will survive its first serious case. A practice in which the answer is no for most of them is not unethical; it is unprotected, and the difference will be discovered by a client.

The first fortnight after qualifying. Build the documents, contract the supervisor, join the group, buy the indemnity. Not because a regulator requires it, but because each of the twelve is a decision the therapist will otherwise make alone, in the moment, with a client waiting.

14.9 Culture, faith and the professional stance

The stance is not culturally neutral, and it does not need to be. The professional obligations described in this book — individual consent, confidentiality against the family, the child's separate interests, the refusal to give legal advice — are the product of a particular history. They are nonetheless the terms on which this profession operates, and a therapist who joins the profession accepts them. The question the student actually faces is not whether the obligations are culturally local but how to hold them inside a communal world without becoming either a foreigner or a hypocrite.

Three ways of holding it.

1. **Translate rather than announce.** Every obligation in this book can be explained in the family's own terms: why an individual session exists, why a record is kept, why a gift of land cannot be accepted, why a report must be made. The therapist who translates is obeying the same rule in the family's language.
2. **Keep the form and the substance separate.** The therapist can eat the food, greet the elder, attend the funeral and say the prayers she says — and still not discuss the client's marriage with the aunt. The forms are the family's; the substance is the profession's.
3. **Be explicit about the limits of your own position.** "I know that in your family the elders would decide this. In my work I am required to let the person decide for herself, and I want to tell you why." Naming the difference is more respectful than either imposing or pretending.

Conscience, and the cases where the therapist's own values will not yield. A therapist may hold a religious conviction about divorce, abortion or same-sex relationships. The professional position is that the conviction is hers and the decision is the client's, and where the therapist cannot provide the service without imposing the conviction, the answer is referral and not persuasion. The two sentences: "I am not the right therapist for this, and I want to tell you that it is about me and not about you", and "here is a name."

Where the law and the client's interest diverge. They do, occasionally: a reporting duty that will harm the child it was designed to protect; a court order that will disclose a client's history to a violent ex-partner; a statutory requirement that the therapist believes is wrong. The professional response is to comply, to say so, to record the objection, and to work for change through the professional body. Civil disobedience is a legitimate act for a citizen and is not available to a clinician in the room, because the person who pays for it is the client.

The closing claim. A stance is not a barrier between the therapist and the family. It is what allows the therapist to be present: the person who has decided what she will not do does not have to decide it again in the session, and the attention that would have gone into that decision goes to the client.

> **DEBATE**
> "A therapist should never impose her values on a client, and a therapist who refers a client because of her own convictions has failed her duty of care." Argue for: the client came for treatment, not judgement, and referral is a rejection that falls hardest on the most vulnerable. Argue against: a therapist who cannot work without imposing is more dangerous in the room than out of it, honesty about limits is a form of respect, and referral with a warm handover is the lesser harm. Then state where your own line is, and what you would say to a client on the other side of it.

14.9a Nothing here is a substitute for the code

One final note on authority. This book has cited principles, statutes and codes, and it is not itself any of them. Where this book and a code disagree, the code governs; where a code and a statute disagree, the statute governs; where a statute and a court order disagree, the order governs. The book's purpose has been to teach the reasoning that makes those authorities usable in a room with a family in it, and a reader who treats it as a rulebook has misread it in the direction of safety, which is at least the right direction.

14.10 Evidence note

What is known. Ethics education research finds that knowledge of codes does not reliably change behaviour, while structured decision supports, checklists and consultation habits do. The literature on clinical judgement documents the gap between self-assessed and observed competence, and the value of feedback and routine outcome measurement. Research on moral distress and moral injury in therapists shows that unaddressed ethical conflict is a major contributor to burnout and to leaving the profession, and that supervision and peer consultation are protective. Studies of decision-making under pressure consistently find that prior commitment — a written policy, a rehearsed sentence — outperforms in-the-moment reasoning.

What is weaker. There is very little evidence on the specific instrument of the stance paper, and none on the seven-step model as a measurable intervention. The claim that they improve practice is derived from the broader literature on implementation and habit rather than from trials of these tools.

What is not supported. That ethical behaviour follows from ethical knowledge. That a therapist's good intentions are protective in the absence of structure. And that experience alone produces ethical judgement — experience without feedback produces confidence, which is a different thing.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same therapist, with the same knowledge, in a practice with supervision and a policy and in a practice without them, is two different clinicians.

14.11 Key terms

Dilemma — a choice between two goods or two harms, as distinct from a difficult situation.

Stance — the set of prior commitments a therapist holds about what she will never do, always consult about, and watch for in herself.

Consultation — seeking the view of another professional on a specific question; distinct from supervision in its form and often in its authority.

Moral distress — the experience of knowing the right action and being prevented from taking it.

Moral injury — the lasting psychological harm that follows acting against one's moral convictions in a professional role.

Implementation intention — a pre-decided response to a specified situation; the mechanism by which policies survive pressure.

Stance paper — the written, dated, annually revised statement of a therapist's commitments and vulnerabilities.

14.11a The one-page summary the course actually asks for

If the fourteen weeks were compressed into a page, it would say six things.

1. Name the client system before you do anything else. Most ethical errors in family work are errors about who the client is.

2. State the frame before it is tested. Consent, confidentiality, its limits, the fee, the cancellation policy, the records. Stated at the start, they are agreements; stated when needed, they are changes of rule.

3. Do not carry what you cannot carry. A secret, a risk, a doubt about competence, an attraction, a debt: each is a signal to consult, and the signal is not a personal failing.

4. Know the four duties that override the frame. Risk to life, risk to a child, statutory reporting, and a court order. Learn them for your jurisdiction, in writing, and review them annually.

5. Write as though it will be read aloud. Because some of it will be.

6. Build the structure that decides the ordinary cases, so that your attention is available for the client in front of you. The structure is not the opposite of relationship; it is what makes relationship possible.

14.12 Self-test

1. State the seven steps, with the failure mode of each.
2. In the Ngozi A. composite, why was option 3 (the forty-eight-hour deadline) rejected?
3. What are the three misuses of the decision model, and how is each detected?
4. Give the three sections of the stance paper and an example of a good entry in each.
5. What are the five things a viva examiner is listening for?
6. What are the three questions asked in some form every time, and why?
7. Give three ways of holding professional obligations inside a communal culture.
8. Why is civil disobedience not available to the clinician in the consulting room?

Answers. (1) Identify the dilemma — naming a situation instead; check codes and law — quoting from memory; consult — consulting the colleague who will agree; generate three options including the uncomfortable one — three versions of the preferred option; weigh consequences for every party — weighing only the client in the room; decide, act, document — documenting the conclusion without the reasoning; evaluate — skipping it. (2) Because it delayed protection for a child in a household where the abuser had access; the deadline was attractive because it preserved the alliance and it subordinated the child's safety to the therapist's discomfort. (3) Justifying a decision already made — detected by a step 4 containing three versions of one option; skipping step 4 — detected

by a binary choice with no uncomfortable option; substituting the model for a stance — detected by a therapist who runs the procedure for every decision and cannot say what she would never do. (4) What I will never do — "I will never write an opinion about a parent I have not assessed"; what I will always consult about — "any case in which I have become the only person the client talks to"; where my values distort me — "I interrupt anger, and I will ask my supervisor monthly whether I have done it". (5) Whose client it is; whether the question can be named without prompting; whether the reasoning is visible; whether knowledge is distinguished from belief; whether a stance is present. (6) "Who was your client?", "what was the ethical question and when did you first notice it?", "what did you not do, and why?" — because they test the three capacities that cannot be faked: defining the system, recognising the question, and reasoning about the rejected option. (7) Translate rather than announce; keep the form and the substance separate; be explicit about the limits of your own position. (8) Because the person who bears the consequence of the therapist's protest is the client, who did not consent to be the occasion of it.

14.13 Reflective task, and the close of the course

Write the five hundred words of the stance paper. Date it. Keep it somewhere you will find it in a year, and when you find it, write the revision: which statements held, which did not, and what belongs in section three now.

Then write one paragraph for yourself alone, not for an examiner, answering this question: what kind of therapist do you intend to be in the situation that frightens you most? The answer will be tested within the first eighteen months. The purpose of fourteen weeks of ethics has not been to make you cautious; it has been to give you a structure you can stand inside when the situation arrives, so that the therapist who is there — tired, fond of the client, unsure — is still recognisably you.

14.14 Further reading

- Beauchamp, T. and Childress, J. on the four principles — the framework used throughout this book, read in the original for its argument rather than its list.
- Kitchener, R. on ethical principles in counselling — the account that underlies the codes' structure.
- Doherty, W. on moral conflicts in family therapy — the best short treatment of the therapist's own values.
- Pope, K. and Vasquez, M. on ethics in psychotherapy and counselling — the standard reference, revised periodically.
- Your professional code, read in full, once a year, with a pencil.
- The literature on moral distress and burnout in therapists — read for the evidence that this subject is a protective factor and not an ornament.

Glossary

Autonomy — the client's right to self-determination. **Beneficence / non-maleficence** — the duty to benefit and the duty not to harm. **Coercive control** — a pattern of domination, isolation and threat, distinct from situational violence. **Confidentiality** — the duty not to disclose information learned in a professional relationship. **Dual relationship** — a second significant relationship with a client or someone close to them. **Duty to warn / protect** — a legal obligation, in some jurisdictions, to act on a serious threat. **Informed consent** — voluntary agreement based on adequate information and capacity. **Mandated reporting** — a legal duty to report suspected abuse or neglect. **Privilege** — the client's legal right to prevent disclosure in court proceedings. **Subpoena** — a legal order requiring documents or testimony. **Welfare principle** — the rule that a child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. In couple therapy, the most defensible approach to individual secrets is: (a) keep everything private (b) declare a policy in advance and apply it (c) tell the partner immediately (d) refuse individual sessions entirely

2. Asking a client directly about suicide: (a) increases risk (b) does not increase risk and is required practice (c) is only for psychiatrists (d) should be avoided in family sessions

3. Couple therapy with a couple in which there is coercive control is: (a) recommended (b) contraindicated until safety is established (c) harmless (d) the only legal option

4. The four principles of biomedical ethics are: (a) autonomy, beneficence, non-maleficence, justice (b) consent, capacity, confidentiality, competence (c) duty, utility, virtue, care (d) respect, honesty, integrity, courage

5. A treating therapist asked to conduct a custody evaluation for their own client should: (a) accept (b) accept if paid (c) decline because of role conflict (d) accept with supervision

6. The welfare principle means decisions about children are governed by: (a) parental preference (b) the child's best interests (c) the court's convenience (d) financial capacity

7. The strongest single predictor of a suicide attempt is: (a) diagnosis (b) previous attempt (c) gender (d) family income

8. A small cultural gift after a bereavement should usually be: (a) refused (b) accepted, discussed and documented (c) accepted silently (d) reported to the police

9. Screening for intimate partner violence should be done: (a) with the couple present (b) with each partner separately (c) by questionnaire only (d) only if asked

10. A defensible ethical decision is one that: (a) everyone agrees with (b) can be explained and documented (c) produces the best outcome (d) follows the client's preference

Answer key

1 b · 2 b · 3 b · 4 a · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. State a confidentiality policy for couple therapy in six sentences a family could understand.
2. List the ten elements of a defensible suicide risk assessment.
3. Distinguish situational couple violence from coercive control, and state the treatment implication.
4. What should you do when you receive a subpoena? Five steps.
5. Why is "I know it is unethical but it was the right thing to do" an inadequate defence?

Essay (choose one, 2,000 words)

1. "The most difficult ethical problems in family therapy are not conflicts between right and wrong but between two rights." Discuss with three cases.
2. A father discloses that his 14-year-old daughter is self-harming and asks you not to tell her mother. Analyse the ethical, legal and clinical dimensions and set out your plan.
3. Critically examine the claim that the professional codes are culturally biased in favour of Western individualism.

Further reading

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). OUP.
- Corey, G., Corey, M. S., & Corey, C. (2019). Issues and Ethics in the Helping Professions (10th ed.).
- Doherty, W. J. (1995). Soul Searching: Why Psychotherapy Must Promote Moral Responsibility. Basic Books.
- Wilcoxon, S. A., Remley, T. P., & Gladding, S. T. (2013). Ethical, Legal and Professional Issues in Counseling (4th ed.). Pearson.
- Child Rights Act (Nigeria), 2003; Violence Against Persons (Prohibition) Act, 2015.

Next: MFT 705 — Research Methods in Marriage and Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 5 OF 28

MFT 705

Research Methods in Marriage and Family Therapy

14 teaching weeks 13 practicals 2 figures

SEMESTER ONE

1.71 units Prerequisite: MFT 701

> You will become a consumer who can read a paper in ten minutes and know what it is worth, and a producer who can design a study you can actually finish.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 705 Research Methods in Marriage and Family Therapy

MSc Marriage and Family Therapy Semester One 1.71 Units 14 teaching weeks Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 705 Research Methods in Marriage and Family Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	**Research Methods in Marriage and Family Therapy**	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 What research is for . . . 6

Week 2 From hunch to question . . . 7

Week 3 Designs overview . . . 7

Week 4 Sampling . . . 9

Week 5 Measurement . . . 9

Week 6 The independence problem . . . 11

Week 7 Quantitative analysis I . . . 12

Week 8 Quantitative analysis II . . . 12

Week 9 Qualitative methods and rigour . . . 13

Week 10 Mixed methods and participatory approaches . . . 13

Week 11 Systematic review and critical appraisal . . . 14

Week 12 Data management . . . 14

Week 13 Research ethics . . . 15

Week 14 Writing it up . . . 15

Glossary . . . 16

Self-assessment . . . 16

- Multiple choice . . . 16
- Answer key . . . 17
- Short answer . . . 17
- Essay (choose one, 2,000 words) . . . 17

Further reading . . . 17

Course leader's summary. You will become a consumer who can read a paper in ten minutes and know what it is worth, and a producer who can design a study you can actually finish. The course is deliberately practical: every week you do something small with real or simulated data.

How to use this book

Research methods are learned by doing badly, then better. Each week has a **concept**, a **worked example**, and a **small task** you can complete in an hour. The tasks build towards your thesis proposal in MFT 770, so keep every one of them.

1. Course description

Family therapy research is harder than individual therapy research for a structural reason: **people in a family are not independent observations**. If you treat a couple as two independent people, you violate the central assumption of most statistical tests. This course teaches you to see that problem and to handle it in ways that survive examiners and journal reviewers.

The course covers: framing a good question; quantitative, qualitative and mixed designs; sampling and measurement; basic analysis; appraisal of evidence; ethics; and writing a proposal.

2. Learning outcomes

#	Outcome	Assessed
LO1	Convert a clinical hunch into a focused, researchable question	Assignment 1
LO2	Select and justify a design appropriate to the question	Exam, proposal
LO3	Evaluate the reliability and validity of commonly used family measures	Exam
LO4	Handle non-independence and unit-of-analysis problems	Exam, proposal
LO5	Interpret basic descriptive and inferential statistics and effect sizes	Exam, data exercise
LO6	Appraise quantitative, qualitative and mixed-methods studies	Appraisal assignment
LO7	Write an ethical, feasible research proposal	Proposal

3. Assessment

Component	Weight	Due
Question and design exercise (1,000 words)	15%	Week 4
Systematic appraisal of two studies (1,500 words)	25%	Week 9

Component	Weight	Due
Data analysis exercise (set problems)	20%	Week 12
Research proposal (2,500 words)	40%	Week 15

4. The fourteen-week plan

Week	Topic	Task
1	What research is for; evidence-based practice and its critics	Find one paper you will use in the thesis
2	From hunch to question: PICO/SPIDER; feasibility	Draft three questions
3	Designs overview: quantitative, qualitative, mixed	Map your question to a design
4	Sampling: who is in, who is out, who never comes	Critique a convenience sample
5	Measurement: reliability, validity, responsiveness	Appraise one couple measure
6	The independence problem: unit of analysis in family data	Worked analysis of couple data
7	Quantitative analysis I: descriptive and comparison	Run descriptives in software
8	Quantitative analysis II: association, prediction, effect size	Interpret a regression table
9	Qualitative methods and rigour	Code a transcript extract
10	Mixed methods; participatory approaches	Design a mixed study
11	Systematic review and critical appraisal	Appraise one meta-analysis
12	Data management, security, documentation	Write a data plan
13	Research ethics and with vulnerable families	Complete an ethics application
14	Writing it up; publication and impact	Proposal presentations

Week 1 What research is for

Three legitimate purposes. Description (what is happening), explanation (why, and under what conditions), and improvement (does this intervention help, for whom). A study that serves none of these is a filing cabinet exercise.

Evidence-based practice is the integration of best available research, clinical expertise, and client values and context. It is not "follow the manual". Two critiques worth knowing:

1. **Efficacy versus effectiveness.** Trials conducted with selected volunteers under ideal conditions may not generalize to your clinic, where families present with debts, migration problems, and court dates.
2. **The evidence hierarchy is question-dependent.** A meta-analysis of trials answers "does it work on average?"; a qualitative study answers "how does it work, and for whom does it fail?". Family therapy needs both.

> **TASK**
> Find one paper relevant to your intended thesis topic. Write its citation and one sentence stating what it establishes and one sentence stating what it leaves open.

Week 2 From hunch to question

A weak question: "Does family therapy work?"

A workable question: "Among adolescents referred to a Lagos community clinic for school refusal, does a six-session family-based intervention reduce days absent from school at 12 weeks, compared with usual care?"

Frameworks. For quantitative work use **PICO** Population, Intervention, Comparison, Outcome. For qualitative work use **SPIDER** Sample, Phenomenon of interest, Design, Evaluation, Research type.

Feasibility screen. Before you fall in love with a design, ask: Do I have access? How many participants can I realistically recruit in six months? Who will approve this? What is the cost? Is the timeline compatible with the semester? Most failed theses are failed at this step, not at analysis.

Common traps.

Trap	Example	Fix
Too broad	"Family functioning in Nigeria"	Specify population, setting, outcome
Unanswerable with your data	"Does family therapy cause lasting change?"	Narrow to what a 12-week design can show
Two questions in one	"Does it work and why?"	One study, one primary question
Question-shaped advocacy	"Why is family therapy better?"	Make it genuinely open ("how does it compare?")

> **TASK**
> Draft three candidate questions. For each, write the PICO or SPIDER elements, the sample you can access, and the main threat to validity.

Week 3 Designs overview

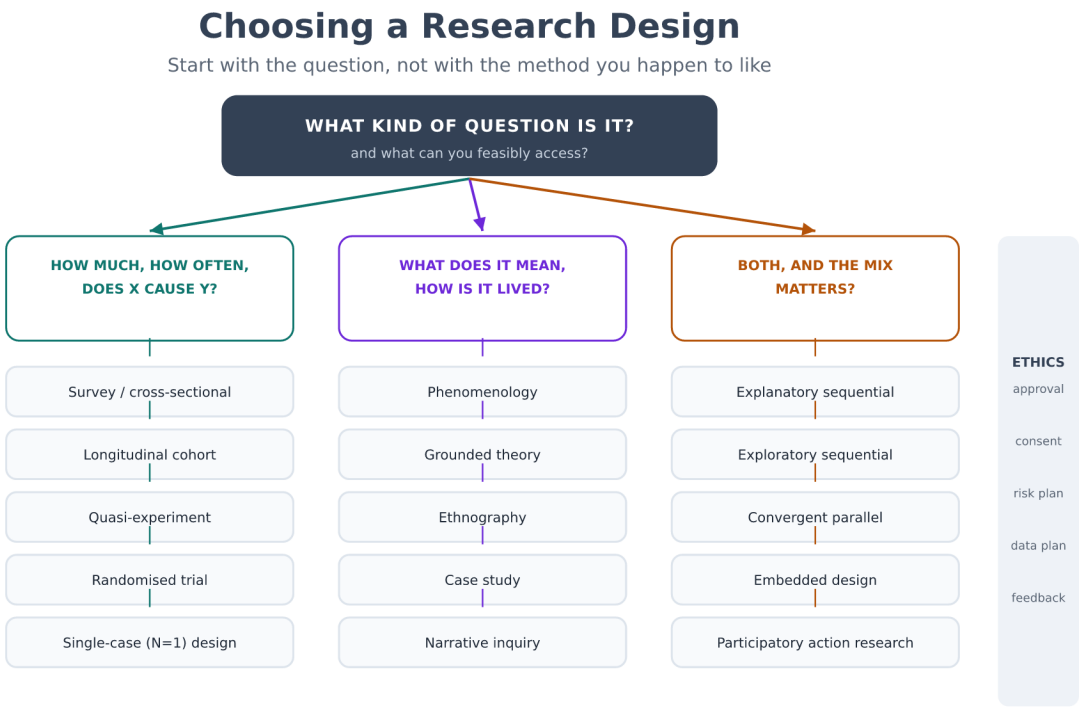


Figure 3.1 Choosing a research design: start with the question

Quantitative.

Design	Answers	Strengths	Threats
Cross-sectional survey	How common? What is associated?	Cheap, fast	Cannot establish temporality
Longitudinal cohort	What predicts change?	Temporal ordering	Attrition
Quasi-experiment	Does the intervention work without randomisation?	Feasible in clinics	Selection bias
Randomised controlled trial	Does it cause the effect?	Strongest causal inference	Cost, ethics, generalisability
Single-case (N=1) design	Does it work for this family?	Clinically realistic	Limited generalisation

Qualitative.

Approach	Focus	Sample size
Phenomenology	Lived experience of a phenomenon	6 12
Grounded theory	Building a theory from data	15 30, to saturation
Ethnography	Culture and practice in a setting	Prolonged fieldwork
Case study	Depth in one bounded system	1 5 cases

Approach	Focus	Sample size
Narrative inquiry	How people story their lives	3 10
IPA	How individuals make sense of experience	3 6

Mixed methods see Week 10.

> **TASK**
> Take your strongest question from Week 2 and map it onto two designs. State what each would let you claim, and what it would not.

Week 4 Sampling

The uncomfortable fact. Clinical samples are almost never representative. Families who attend are different from families who are referred and do not attend, and those are different again from families who are never referred. Your findings describe attenders, not families.

Probability sampling (random, stratified, cluster) permits generalisation. **Non-probability sampling** (convenience, purposive, snowball, quota) does not and this is fine, if you say so.

In family research specifically:

- **Who is the participant?** Mother, father, child, whole family, or the therapist's rating of the family?
- **Who refuses?** Fathers are the most common absence in family research and in family therapy. A study that reports "family functioning" from mothers' reports is a study of mothers.
- **Sample size.** For quantitative work, do a power calculation before you collect (software: G*Power, or the pwr package in R). For qualitative work, plan for saturation and state how you will judge it.

> **TASK**
> Critique the sample in any published Nigerian family study: who was included, who was excluded, and what does the exclusion do to the conclusions?

Week 5 Measurement

A measure must be **reliable** (consistent), **valid** (measuring what it claims), and **responsive** (sensitive to change).



Figure 5.1 Four validity targets and three reliability methods

The four validity targets.

Validity	The question
Internal	Did the intervention, and not something else, produce the change?
External	Can these findings travel to other families, settings and cultures?
Construct	Are you measuring "alliance", or "wanting to please the therapist"?
Statistical	Is the effect large enough, and the test powerful enough, to trust?

Instruments you should know (and check for local validation):

Instrument	Measures	Notes
Dyadic Adjustment Scale (DAS) / DAS-4	Relationship satisfaction	Widely used; short forms available
Revised Dyadic Adjustment Scale (RDAS)	Satisfaction	14 items
FACES-IV	Cohesion and flexibility	Psychometrics debated; use cautiously
Family Assessment Device (FAD)	Family functioning (McMaster)	60 items; subscales
Working Alliance Inventory (WAI-SR)	Therapeutic alliance	Adapt for family work carefully
PHQ-9 / GAD-7	Depression / anxiety	Free, brief, widely validated

Instrument	Measures	Notes
SDQ	Child emotional and behavioural functioning	Parent, teacher, self versions

The cultural validity question. Was this instrument developed and validated with families like the ones you see? If not, report that limitation and consider a local adaptation study (a perfectly good thesis).

> **TASK**
> Take one couple measure. Find two published validations, one in an African sample if it exists. Report: internal consistency, factor structure, and any evidence of measurement invariance.

Week 6 The independence problem

This is the technical heart of family research.

The problem. In a study of 100 couples, you do not have 100 independent observations; you have 50 couples, and partners' scores are correlated. Standard tests (t-test, ANOVA, OLS regression) assume independence. Violating it inflates Type I error: you will find effects that are not there.

Three solutions.

1. **Analyse at the dyad level.** Compute a couple score (mean, difference, or both) and analyse that. Simple, loses information about within-couple differences.
2. **Treat the dyad as the unit and model non-independence explicitly.** Multilevel models (HLM) with individuals nested in couples nested in therapists. This is the modern standard.
3. **Dyadic models.** The **Actor Partner Interdependence Model (APIM)** separates actor effects (my predictor on my outcome) from partner effects (my predictor on my partner's outcome). This is often the clinically interesting question.

Worked example. Suppose you measure criticism (predictor) and satisfaction (outcome) in 60 couples. An OLS regression treating all 120 people as independent finds criticism predicts satisfaction, $p = .001$. An APIM re-analysis finds: actor effect significant; partner effect smaller but present; and the standard errors widen. The clinical interpretation changes: criticism damages the person who gives it as well as the person who receives it.

> **TASK**
> Simulate it: in a spreadsheet, create 40 couples with correlated scores. Run a t-test treating all 80 as independent, then compute the couple means and run the test again. Note the difference in p.

Week 7 Quantitative analysis I

Descriptive statistics. Mean and standard deviation for symmetric data; median and interquartile range for skewed data (income, session counts). Always report n, missingness, and the range.

Comparing groups. t-test (two groups), ANOVA (three or more), chi-square (categorical). Check assumptions: independence, normality (roughly), equal variance.

Clinical significance versus statistical significance. A drop of 2 points on a satisfaction scale can be statistically significant with n = 400 and clinically meaningless. Report **effect sizes** and, where available, the **minimal clinically important difference**.

Common effect sizes.

Statistic	Meaning	Rough benchmark
Cohen's d	Standardised mean difference	0.2 small, 0.5 moderate, 0.8 large
Pearson's r	Association	0.1 small, 0.3 moderate, 0.5 large
Odds ratio	Relative odds	1.5 modest, 2+ substantial
Number needed to treat	How many treated for one extra success	Lower is better

> **TASK**
> Interpret this sentence for a non-researcher in two sentences: "The intervention group showed a greater reduction in distress than controls (d = 0.55, 95% CI [0.21, 0.89], p = .002)."

Week 8 Quantitative analysis II

Correlation is not causation, and neither is regression. Regression tells you how much of the variance in an outcome is associated with predictors, adjusted for other predictors. It does not tell you the direction, unless the design does.

Reading a regression table. Look at: which predictors are significant; the sign and size of the coefficient (in meaningful units); the confidence intervals; R^2 (variance explained); and whether the model diagnostics were reported.

Moderation and mediation. Moderation: for whom, or under what conditions, does the effect hold? (e.g. does the intervention work better for younger adolescents?) Mediation: through what mechanism? (e.g. does improved communication explain the reduction in distress?). Mediation requires strong assumptions and, ideally, longitudinal data.

> **TASK**
> Take a published regression table from a family study. Write four sentences a clinician could use, including one caveat.

Week 9 Qualitative methods and rigour

When qualitative methods are the right choice: when you need to know how something is experienced, why a service is not used, what a concept means locally, or how a therapy unfolds. They are not a fallback for people who dislike numbers.

Rigour criteria (Lincoln & Guba; Braun & Clarke for reflexive TA):

Criterion	Practical meaning
Credibility	Do the findings ring true to participants? (member checking, triangulation)
Transferability	Enough context for a reader to judge relevance elsewhere (thick description)
Dependability	An audit trail of decisions
Confirmability	Findings grounded in data, not in the researcher's preferences
Reflexivity	Your own position examined, not hidden

Coding, briefly. Familiarise code generate themes review define write. Coding can be inductive (from the data) or deductive (from a framework). Reflexive thematic analysis explicitly treats themes as constructed by the analyst, not excavated from the data.

Sample extracts. Suppose a father says: "They asked me to come, but nobody asked me what I thought was wrong. I sat there like a visitor in my own house." Codes: excluded, not consulted, home as foreign. Candidate theme: "A visitor in my own house": the disengagement of fathers in family therapy. Note how the theme uses the participant's own language.

> **TASK**
> Code the extract above independently, then compare with a peer. Where you disagree, ask whether you are coding the same unit.

Week 10 Mixed methods and participatory approaches

Why mix. Numbers answer "how much"; words answer "how" and "why". Three designs:

1. **Explanatory sequential** quantitative first, then qualitative to explain the result.
2. **Exploratory sequential** qualitative first (to build a culturally valid measure or model), then quantitative.
3. **Convergent parallel** both at once, integrated in interpretation.

Integration is the hard part. A study that reports a survey and some interviews side by side, without integrating the findings, is two studies in one paper. Use a **joint display** table: finding from strand A

| finding from strand B | convergence / divergence | interpretation.

Participatory action research is particularly well suited to community and family work: families help define the question, collect data, and interpret findings. It strengthens validity and ethics at once.

> **TASK**
> Design a mixed-methods study to understand why families drop out after the first session. Specify the sequence, the integration point, and one joint display row.

Week 11 Systematic review and critical appraisal

Levels of evidence (roughly, for treatment questions): systematic reviews and meta-analyses > RCTs > quasi-experiments > cohort > case-control > cross-sectional > case series > expert opinion. For questions of meaning, the hierarchy inverts.

Appraising a meta-analysis. Ask: Was the search systematic and pre-registered? Were studies comparable enough to pool? Was heterogeneity (I) reported and explained? Was publication bias assessed (funnel plot, Egger's test)? Do the authors' conclusions follow from the data?

A ten-minute paper reading routine.

1. Abstract: design, n, outcome, conclusion.
2. Participants: who, where, how recruited.
3. Comparison: what was the control?
4. Measure: does it measure something you care about?
5. Effect: size and confidence interval, not just p.
6. Limitations: what the authors admit and what they do not.
7. Relevance: your population, your setting, your resources.

> **TASK**
> Appraise one meta-analysis of couple or family therapy. Write 500 words: what it establishes, what it cannot establish, and one clinical implication.

Week 12 Data management

Principles. Collect the minimum you need; store it encrypted; separate identifiers from data; define retention and destruction; document every decision.

Practical protocol for a master's thesis.

Element	Rule
Identifiers	Separate key file, encrypted, access limited to you and supervisor
Audio/video	Encrypted drive, deletion date agreed with participants

Element	Rule
Transcription	Pseudonymised at the point of transcription, not after
Version control	Date every dataset; never overwrite raw data
Analysis log	One page per week: what you did, what you decided, why
Backup	Two locations, one offline

> **TASK**
> Write a one-page data management plan for your proposed study.

Week 13 Research ethics

Core requirements. Institutional ethics approval before recruitment; informed consent appropriate to age and capacity; voluntariness (especially for your own clients); protection of confidentiality; a plan for distress and incidental findings; fair selection of participants; and a dissemination plan that returns something to the community.

Specific issues in family research.

- **Consent within families:** if a parent consents, does the adolescent have a genuine choice? Use separate assent and consent processes.
- **Coercion by proxy:** recruiting your own clients creates pressure. Use a third party for recruitment.
- **Group confidentiality:** you cannot guarantee that family members will not repeat what is said.
- **Power and payment:** paying participants can be appropriate compensation or an undue inducement; be transparent about the reasoning.

> **TASK**
> Complete a mock ethics application for your proposal, including consent forms for an adult and for a 14-year-old.

Week 14 Writing it up

Structure (IMRaD). Introduction (why this question matters; what is known; the gap) Method (what you did, in enough detail to replicate) Results (what you found, without interpretation) Discussion (what it means, limitations, implications) Conclusion.

Writing rules that examiners reward. One idea per paragraph; active voice where possible; report numbers with units and confidence intervals; state limitations honestly and specifically; do not overclaim in the abstract.

Publication. Target a journal that publishes practitioner research; follow the author guidelines exactly; expect rejection once and revise.

Glossary

Actor partner interdependence model (APIM) a dyadic model separating one's own and one's partner's effects. **Attrition** loss of participants over time. **Confidence interval** the range within which the true value is likely to fall. **Construct validity** the degree to which a measure captures the intended concept. **Effect size** the magnitude of an effect, independent of sample size. **Heterogeneity (I)** the proportion of variation across studies due to real differences rather than chance. **Independence assumption** the statistical requirement that observations are not correlated. **Measurement invariance** whether an instrument measures the same construct across groups. **Power** the probability of detecting an effect that exists. **Saturation** the point at which new data yield no new themes. **Triangulation** using multiple methods, sources or analysts to strengthen inference.

Self-assessment

Multiple choice

1. Treating both partners in 50 couples as 100 independent observations primarily threatens: (a) external validity (b) statistical conclusion validity (c) construct validity (d) reliability

2. The APIM distinguishes: (a) pretest and posttest (b) actor and partner effects (c) randomisation and control (d) qualitative and quantitative

3. A qualitative study of lived experience with 8 participants is most likely: (a) grounded theory (b) phenomenology (c) ethnography (d) RCT

4. Cronbach's alpha estimates: (a) test retest reliability (b) inter-rater reliability (c) internal consistency (d) construct validity

5. The strongest design for causal inference is: (a) cross-sectional survey (b) quasi-experiment (c) randomised controlled trial (d) case series

6. "For whom and under what conditions does the effect hold?" refers to: (a) mediation (b) moderation (c) randomisation (d) sampling

7. Which is not a qualitative rigour criterion? (a) credibility (b) transferability (c) generalisability (d) reflexivity

8. A 95% confidence interval that includes zero suggests: (a) a large effect (b) a non-significant effect at the .05 level (c) a reliable measure (d) a sampling error in the measure

9. The most common absence in family research samples is: (a) mothers (b) fathers (c) adolescents (d) grandparents

10. Reflexivity in qualitative research means: (a) the researcher examines their own position and effect (b) participants review findings (c) the study is replicated (d) data are triangulated

Answer key

1 b 2 b 3 b 4 c 5 c 6 b 7 c 8 b 9 b 10 a

Short answer

1. Convert "does family therapy help adolescents?" into a PICO question.
2. Explain the unit-of-analysis problem in one paragraph a clinician would understand.
3. Name three threats to internal validity in a pre-post clinic study.
4. What is measurement invariance and why does it matter in cross-cultural family research?
5. List five elements of a data management plan.

Essay (choose one, 2,000 words)

1. "The randomised controlled trial is the wrong gold standard for family therapy." Discuss.
2. Design a mixed-methods study of therapy drop-out in a Nigerian family clinic, justifying every methodological choice.
3. Critically appraise the evidence that common factors explain most psychotherapy outcome.

Further reading

- Kazdin, A. E. (2021). Research Design in Clinical Psychology (5th ed.). CUP.
- Wampold, B. E., & Imel, Z. E. (2015). The Great Psychotherapy Debate (2nd ed.). Routledge.
- Kenny, D. A., Kashy, D. A., & Cook, W. L. (2006). Dyadic Data Analysis. Guilford.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Snyder, D. K., Heyman, R. E., & Haynes, S. N. (2005). Evidence-based approaches to assessing couple distress. Psychological Assessment, 17(3), 288 307.
- Lebow, J., & Snyder, D. K. (2022). Clinical Handbook of Couple Therapy (6th ed.). Guilford.

Next: MFT 706 Clinical Practice and Techniques

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 6 OF 28

MFT 706

Clinical Practice and Techniques

16 teaching weeks 10 practicals 4 worked cases 1 figure

SEMESTER ONE

1.71 units Prerequisite: MFT 701, 702

> This is the skills course.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 706 Clinical Practice and Techniques

MSc Marriage and Family Therapy Semester One 1.71 Units 16 teaching weeks Prerequisite: MFT 701, 702

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 706 Clinical Practice and Techniques. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	**Clinical Practice and Techniques**	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The sixteen-week plan . . . 6

Week 1 The architecture of a session . . . 6

Week 2 Joining . . . 7

Week 3 Listening . . . 7

Week 4 Questioning I . . . 8

Week 5 Questioning II . . . 9

Week 6 Enactment . . . 10

Week 7 Reframing, normalising, positive connotation . . . 10

Week 8 Observed session 1 . . . 11

Week 9 Working with conflict and escalation . . . 11

Week 10 Silence, tears, and the therapist's discomfort . . . 12

Week 11 Structuring the room and the system . . . 12

Week 12 Between-session tasks . . . 13

Week 13 Impasses, non-attendance and drop-out . . . 14

Week 14 Working with children and adolescents in family sessions . . . 14

Week 15 Endings . . . 14

Week 16 Observed session 2 . . . 15

Glossary . . . 16

Self-assessment . . . 16

- Multiple choice . . . 16
- Answer key . . . 16
- Short answer . . . 17
- Essay (choose one, 1,800 words) . . . 17

Further reading . . . 17

Course leader's summary. This is the skills course. You will leave able to structure a session, ask six kinds of question deliberately, conduct an enactment, reframe without dismissing, assign a task that gets done, and end therapy well. Most of the hours are practice, not lecture.

How to use this book

Technique is a set of **deliberate choices**, not a personality. Each week introduces one family of skills, gives you the language, shows a transcript, and then requires you to perform it. Read the transcript aloud before the lab: your mouth needs to know the sentences before your mind can choose them under pressure.

1. Course description

The gap between knowing about family therapy and doing it is bridged by rehearsal. This course covers the micro-skills (joining, questioning, reflecting, summarising), the meso-skills (structuring a session, managing conflict, conducting an enactment), and the macro-skills (planning treatment, assigning tasks, handling impasse, terminating).

It is assessed almost entirely on **performance**, because a student who can describe an enactment and cannot conduct one is not yet a clinician.

2. Learning outcomes

#	Outcome	Assessed
LO1	Structure a session with a clear opening, middle and close	Observed session
LO2	Use six categories of question deliberately and appropriately	Observed session, exam
LO3	Conduct an enactment and manage its escalation	Observed session
LO4	Reframe, normalise and positively connote without dismissing distress	Skills test
LO5	Design and review between-session tasks	Task portfolio
LO6	Manage conflict, silence and resistance in the room	Observed session
LO7	Plan and conduct a planned ending	Viva, log

3. Assessment

Component	Weight	Due
Skills triad log (weekly)	20%	Fortnightly
Observed session 1 (20 min, mid-semester)	20%	Week 8
Task portfolio (5 designed tasks with rationale)	20%	Week 12

Component	Weight	Due
Observed session 2 (40 min, with feedback)	40%	Week 16

4. The sixteen-week plan

Week	Skill family	Practical
1	The architecture of a session	Structure a 50-minute session
2	Joining and the first five minutes	Opening rounds, three styles
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Questioning I: linear, circular, strategic	Circular questioning drill
5	Questioning II: reflexive, future-oriented, scaling	Solution-focused drill
6	Enactment: making the family talk to each other	Enactment lab
7	Reframing, normalising, positive connotation	Reframe tennis
8	**Observed session 1**	Assessment and feedback
9	Working with conflict and escalation	De-escalation drill
10	Silence, tears, and the therapist's discomfort	Tolerance-of-silence lab
11	Structuring: who sits where, who speaks when, subgroups	Subgroup session design
12	Between-session tasks and homework	Task portfolio workshop
13	Impasses, non-attendance, and drop-out	Difficult-session rehearsal
14	Working with children and adolescents in family sessions	Live supervision
15	Endings: planned, unplanned, and therapeutic	Termination role-play
16	**Observed session 2**	Full assessment

Week 1 The architecture of a session

A 50-minute family session has five movements. Beginners skip the first and the last, which is why their sessions feel formless.

Movement	Minutes	Purpose
1. Opening / re-joining	3 5	Reconnect, state the frame, ask what has changed
2. Agenda	2 3	Whose concern is most urgent this week?
3. Work	25 30	The core intervention: enactment, questioning, restructuring
4. Consolidation	5 7	Summarise, check, agree the task
5. Close	3 5	Confirm next appointment, name one thing to watch

The "what has changed?" opener. Never start where you left off. Start with the family's own observation: "Before we begin what has been different this week, even slightly?" This trains the family to notice change and gives you data about what is already working.

> **PRACTICAL**
> Write the five movements on a card. Use it in every triad this semester. By Week 8 you should not need it.

Week 2 Joining

Three joining styles (choose deliberately):

1. **Tracking** follow the family's language and pace. "Tell me about 'stubborn' what does it look like in your house?"
2. **Accommodation** adopt the family's structure for the moment. With a hierarchical family, address the father first; with a chaotic family, bring structure immediately.
3. **Personal joining** offer a small, true piece of yourself. "I grew up in a house where nobody raised their voice either. It took me years to learn to argue."

What joining is not. It is not agreement, flattery, or becoming the family's friend. You can join and disagree: "I like the way you two can laugh at this. I am going to say something you may not like."

Three failures of joining.

Failure	Symptom	Repair
Over-joining	You are now part of the family's pattern	Name it: "I think I just took your side. Let me step back."
Under-joining	Family answers in monosyllables	Slow down; ask about something concrete and safe
Premature confrontation	Family defensive from minute five	Return to their account for ten minutes before challenging

> **PRACTICAL**
> Three rounds of five-minute openings, each with a different joining style. The "family" rates: did I feel understood? Did I feel patronised? Did I want to continue?

Week 3 Listening

Paraphrase return the content. **Reflect** return the feeling. **Summarise** return the pattern across people and time.

Client: "He comes home, eats, and goes to the room. I sit in the parlour with the children and I feel like a widow."

- Paraphrase: "So most evenings he is in the room and you are with the children."
- Reflect: "You feel alone in your own marriage."
- Summarise (later, across people): "So there are two versions of the evening: yours is that he leaves; his is that he arrives already exhausted and afraid of saying the wrong thing. Both of you are alone in the same house."

The 2:1 rule for beginners. Two reflections for every question. It slows the interview, produces richer material, and prevents the interrogation pattern families experience in first sessions.

> **PRACTICAL**
> Ten-minute interview in which you may not ask a single question. Only paraphrase, reflect and summarise. Note how uncomfortable it is, and what the client does when you stop asking.

Week 4 Questioning I

Six categories, each with a purpose. Learn the label so you can choose under pressure.

Type	Purpose	Example
Linear	Gather facts	"When did the arguing start?"
Circular	Reveal difference and relationship	"Who notices first when your mother goes quiet?"
Strategic	Introduce a direction	"What would happen if you stopped reminding him?"
Reflexive	Invite a new perspective	"If your daughter could hear this conversation, what would she say you both believe?"
Future-oriented	Open possibility	"Suppose this is better in six months what would be different?"
Scaling	Make change measurable	"On 0 10, how close are you as a couple? What would one point look like?"

The circular question is the field's signature skill. Build it from three elements: a difference (who/what/how much), a relationship (between whom), and a comparison (more than, less than, compared with).

- Weak: "Is your husband supportive?"
- Better: "Who in the family would say your husband is most supportive and who would disagree?"
- Best: "If your mother were watching the two of you last night, what would she say was different between how he spoke to you and how he speaks to your sister?"

> **PRACTICAL**
> Circular questioning drill: 10 minutes, all questions must be circular. Observer counts and classifies. Target: 80% genuinely circular.

Week 5 Questioning II

Reflexive questions invite the family to observe itself. They are the safest way to raise something difficult, because the family supplies the content:

- "What do you think your son has learned about marriage from watching the two of you?"
- "If I asked your daughter who is in charge in this house, what would she say?"
- "What is the most useful thing that could happen in this session and the most uncomfortable?"

Future questions create hope without denying the present:

- "Suppose we meet in a year and this is behind you. What would you want to be able to say you did?"
- "What would your father have wanted for this family?"

The question to avoid: "Why?" in relation to behaviour. "Why did you shout at her?" invites defence or invention. "What happened just before you shouted?" invites sequence.



Figure 5.1 Solution-focused moves: strengths, miracle, scaling, exceptions, task

> **PRACTICAL**
> Convert ten "why" questions into "what/how/who" questions. Compare the answers you get in role-play.

Week 6 Enactment

Definition. Enactment is the deliberate direction of family members to speak **to each other** in the session, while the therapist moves to the edge of the action.

Why it matters. Most families come to therapy to talk about each other to a third party. Enactment converts description into live interaction, where you can observe and shape it. It is the single most powerful structural technique.

The four-step protocol.

1. **Block the detour:** "Don't tell me tell her."
2. **Specify the content:** "Tell her what you needed on Saturday, in one sentence."
3. **Direct the process:** "Look at him, not at me." "Say it again, slower."
4. **Intensify or interrupt:** "You just smiled. Say it again with the smile." Or: "Stop. What just happened?"

Transcript (structural enactment with a couple).

> **THERAPIST:**
> You have both told me about Saturday. Now I want you to tell each other. Funmi, tell Femi what you needed on Saturday one sentence, and look at him. **Funmi:** (to therapist) I needed him to **Therapist:** To him. **Funmi:** I needed you to call and say you would be late. **Femi:** I did call **Therapist:** Femi, answer her, not me. **Femi:** (pause) I did call. At four. You didn't pick up. **Funmi:** I was with your mother. **Therapist:** Stop there. Funmi what happened inside you when he said that?

Hazards. Enactment can escalate. Have a stop signal, keep your voice calm and slow, and interrupt before contempt. With coercive control, do not enact conflict; safety first (MFT 704).

> **PRACTICAL**
> Enactment lab. Triads with a scripted couple. The therapist may only use the four steps. Observer records: did the therapist stay out of the content? Did the family speak to each other for more than 90 seconds?

Week 7 Reframing, normalising, positive connotation

Reframing changes the frame, not the facts. It is the most-used intervention in the field and the most easily abused.

Original frame	Reframe	Risk
"He is controlling"	"He is frightened of losing you and tries to hold on"	Can excuse control
"She is manipulative"	"She has learned that tears are the only way to be heard"	Can remove accountability
"You are lazy"	"You are exhausted and have stopped asking for help"	Can be heard as excusing
"Our son is defiant"	"Your son is trying to protect his mother"	Can burden the child further

Three tests before you reframe.

1. **Is it true enough?** A reframe that the family rejects as false damages alliance.
2. **Does it open or close action?** "He is frightened" opens action; "he is a monster" closes it.
3. **Does it remove accountability?** Reframes should increase the family's sense of agency, not reduce it.

Normalising placing behaviour in a developmental or statistical context: "Arguing about money is the most common argument in the first five years of marriage. It is not a sign that you are failing."

Positive connotation (Milan) describing all behaviour as serving the family system: "Your arguments may be the family's way of ensuring that nothing important is decided without both of you being fully present." Use sparingly; it can sound like sarcasm.

> **PRACTICAL**
> "Reframe tennis": one student states a complaint, the partner must return three successive reframes, and the first student says which one she would accept and why.

Week 8 Observed session 1

Twenty minutes with a role-played family. Assessed against five criteria: structure (does the session have a shape?), joining (does each person feel heard?), questioning (at least three circular), one enactment, and a summary with a task.

Feedback is given live by the observing group using the structured sheet, then by the lecturer.

Week 9 Working with conflict and escalation

The therapist's job during conflict is regulation, not resolution. Five moves:

1. **Slow the pace.** "Stop. Say that again, more slowly."
2. **Name the process.** "I notice that every time she raises her voice, you leave the room. Can we stay with it for two minutes?"

3. **Structural interruption.** "I am going to ask you both to be quiet for one minute and let me speak to your son."
4. **Separate content from process.** "The topic is money. The pattern is that you both start explaining before the other finishes."
5. **Time-out with a return.** "Let's take two minutes. We are not finished."

Escalation signs to interrupt early: raised volume, contempt (eye-rolling, mockery), one person standing, threats, children becoming still or leaving, your own heart rate rising.

> **DE-ESCALATION DRILL**
> Pairs escalate a scripted argument on cue. The therapist must interrupt before the third escalation. Repeat until the interruption happens at the right moment, not the safe one.

Week 10 Silence, tears, and the therapist's discomfort

Silence is a technique. There are four kinds:

Silence	Meaning	Response
Processing	Someone is thinking	Wait. Count to fifteen.
Emotional	Feeling has arrived	Stay; soften your face; do not rescue
Resistance	Something is not being said	Name it gently: "Something just shifted. What happened?"
Your own	You are lost	Say so: "Let me think for a moment about what I am hearing."

Tears. Do not hand a tissue immediately; it tells the person to stop. Wait three seconds, then offer. Ask: "Is this a sadness that has been waiting a long time?"

Your discomfort is data. If you want to fill the silence, ask what you are avoiding. If you feel irritated with one family member, ask what they represent to you. This is the beginning of using the self as an instrument (developed in MFT 720 and 722).

Week 11 Structuring the room and the system

Physical structure. Seat families so that coalition patterns are visible and can be interrupted. Put the parents together and the children together; sit between people who never speak; never sit in a line that recreates the hierarchy you are trying to change.

Who attends. Deliberately choose the combination: whole family, parents only, siblings, one parent and one child, individual. State the rationale: "Next week I would like to see the two of you alone, so you can talk as a couple without the children carrying it."

Subgroup sessions. A powerful and under-used technique: see the parents alone to plan a united message, then bring the children in and have the parents deliver it together. The intervention is the difference between the two sessions.

PRACTICAL

Design a three-session sequence using different attendance combinations for the Adeyemi family (MFT 701). Justify each combination.

Week 12 Between-session tasks

A task is a test of the system, not homework. It should be small, specific, behavioural, and designed to produce information.

The four-part task formula.

1. **What exactly** ("For the next seven days, at dinner ")
2. **Who does what** (" each of you names one thing that went well in your day ")
3. **What to notice** (" and notice what happens to the atmosphere by Thursday.")
4. **What to bring back** (" we will look at what you noticed next week.")

Five useful task types.

Type	Purpose	Example
Observation	Gather data	"Notice and write down every time the argument stops before it starts."
Ritual	Interrupt a sequence	"One night this week, the parents go out for one hour. The children are told in advance."
Behavioural exchange	Build positives	"Three small caring acts each per day, unannounced."
Communication	Practise a skill	"Two 15-minute speaker listener conversations, timer on."
Paradoxical (rare, supervised)	Change the symptom's function	"On Tuesday, have the argument on purpose for ten minutes at 8 p.m."

Why tasks fail. Too big, too vague, agreed under pressure in the last two minutes of the session, or the family does not believe the therapist believes they will do it. Write the task down and give them the paper.

TASK PORTFOLIO (20%)

Five tasks, each with: the presenting problem, the task, the rationale, the predicted failure mode, and the review question you would ask.

Week 13 Impasses, non-attendance and drop-out

When a session goes flat. Three likely causes: the family is protecting a secret; the therapist has joined too closely to one side; or the work has reached material the family did not consent to. Name the process: "I notice we have been very polite for twenty minutes. What are we all avoiding?"

When someone does not attend. Never pretend it did not happen. "Your father chose not to come today. Let's think about what that means." Explore whether attendance is a form of communication.

When the family drops out. Follow up once, by phone, briefly and without pressure; record the reason if given; and use it as clinical data. In family therapy, drop-out is the most common negative outcome, and it is usually visible early the first two sessions predict the rest.

Impasses with the therapist. If you are bored, angry, or over-involved, that is information about the system. Take it to supervision before you act on it.

Week 14 Working with children and adolescents in family sessions

Practical adjustments.

- Shorter sessions for younger children (30 40 minutes), or split sessions.
- Explain the frame to the child directly, not only to the parents: "In this room we talk, and sometimes we draw."
- Give the child a legitimate role: "You are the expert on what happens at school."
- Protect the child from being made the messenger: "That is a question for your mother to answer, not you."
- Never ask a child to choose sides in front of the parents.

The adolescent rule. Speak to the adolescent as the primary person for at least five minutes. Ask their version first. Parents' accounts of adolescents are systematically more negative than adolescents' own reports and than teachers' reports.

> **LIVE SUPERVISION**
> One student conducts a family session with a trained role-play family while the group observes; the supervisor may call in with one suggestion. Debrief with the family's experience.

Week 15 Endings

Planned ending. Agree it in advance; review what changed and what did not; name the risk of relapse; write a one-page relapse plan with the family; leave the door open.

The four questions of a good ending.

1. What is different now, in your own words?
2. What did each of you do that made the difference?
3. What is most likely to make it come back?
4. What will you do when you notice the first sign?

Unplanned endings. Client-initiated endings are common; acknowledge and, where possible, send one closing message. Therapist-initiated endings (illness, placement change, moving) require a proper handover session an abrupt ending can replicate an abandonment for the family.

Do not adopt. A warm ending can slip into a friendship. Say what the relationship was: "I have enjoyed working with you. This has been a professional relationship, and it ends here that is what makes it safe."

> **PRACTICAL**
> Termination role-play: a family that has improved and is anxious about stopping. The therapist must review, name the risk, write the plan and close, in 15 minutes.

Week 16 Observed session 2

Forty minutes, full assessment, using the criteria below and a self-assessment submitted in advance.

Criterion	Weight
Structure and pacing	20%
Joining and alliance with each member	20%
Questioning (range and timing)	20%
One deliberate intervention well executed	25%
Summary, task and closing	15%

Glossary

Accommodation adapting one's style to a family's organisation in order to join it. **Circular question** a question about difference or relationship that reveals pattern. **Enactment** directing family members to interact with each other in session. **Intensity** raising the emotional or interactional stakes to produce change. **Positive connotation** framing all behaviour as serving the family system. **Reflexive question** a question inviting the family to observe itself. **Reframe** relabelling behaviour within a new, usually relational, frame. **Subgroup session** seeing part of the family deliberately, then using the difference. **Unbalancing** deliberately supporting one member to shift the system's equilibrium.

Self-assessment

Multiple choice

1. The correct response to "she is manipulative" as a first formulation is: (a) agree (b) challenge the parent (c) gather specific behavioural sequences (d) diagnose

2. "Who notices first when your mother goes quiet?" is: (a) linear (b) circular (c) strategic (d) closed

3. The main purpose of enactment is to: (a) gather history (b) observe and shape live interaction (c) calm the family (d) demonstrate expertise

4. A reframe should: (a) excuse the behaviour (b) open new action without removing accountability (c) always be positive (d) replace diagnosis

5. Two reflections for every question is recommended because it: (a) fills time (b) reduces interrogation and enriches material (c) prevents emotion (d) shortens sessions

6. A task is most likely to fail when it is: (a) written down (b) small and specific (c) agreed in the last two minutes (d) behavioural

7. Therapeutic silence after an emotional statement should usually be: (a) filled quickly (b) held for 10 15 seconds (c) ended by offering a tissue (d) followed by a question

8. In a family with coercive control, enactment of conflict is: (a) recommended (b) contraindicated (c) neutral (d) required

9. Asking "why did you shout?" is discouraged because it invites: (a) facts (b) defence or invention (c) circular information (d) emotion

10. A good ending includes: (a) a new diagnosis (b) a relapse plan (c) a friendship (d) a referral to another therapist

Answer key

1 c 2 b 3 b 4 b 5 b 6 c 7 b 8 b 9 b 10 b

Short answer

1. Write one example of each: linear, circular, reflexive and scaling question, on the same topic.
2. Give the four steps of enactment with the exact words you would use for step 1.
3. What are the three tests of a good reframe?
4. Name four reasons between-session tasks fail.
5. How would you structure a session for a family with a 7-year-old and a 15-year-old?

Essay (choose one, 1,800 words)

1. "Technique is the servant of the alliance." Discuss with reference to your own observed sessions.
2. Analyse a transcript of your own session and identify three moments where a different choice was available. Justify each alternative with theory.
3. How should a family therapist adapt technique for a family in which the father is structurally absent?

Further reading

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Tomm, K. (1988). Interventive interviewing: Part III. Family Process, 27(1), 1 15.
- Hepworth, M., & Larsen, J. (1993). Direct Social Work Practice. Brooks/Cole. (Micro-skills chapters)
- Butler, C., & Pilling, S. (2020). Competence Framework for Systemic Family and Couple Therapy. UCL.
- Pilling, S., & Fornells-Ambrojo, M. (2015). Supervision and Consultation. UCL Competence Frameworks.

Next: MFT 707 Assessment and Diagnosis in Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 7 OF 28

MFT 707

Assessment and Diagnosis in Family Therapy

15 teaching weeks 6 practicals 3 figures

SEMESTER ONE

1.71 units Prerequisite: MFT 701, 703

> You will learn to gather multi-informant data, use standardised measures properly, formulate relationally, use diagnosis without being used by it, and communicate all of it to a family in words

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 707 Assessment and Diagnosis in Family Therapy

MSc Marriage and Family Therapy Semester One 1.71 Units 15 teaching weeks Prerequisite: MFT 701, 703

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 707 Assessment and Diagnosis in Family Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	**Assessment and Diagnosis in Family Therapy**	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 Purposes of assessment . . . 6

Week 2 The multi-informant principle . . . 7

Week 3 History taking . . . 8

Week 4 Interaction observation . . . 9

Week 5 Standardised measures I: relationship and family functioning . . . 9

Week 6 Standardised measures II: symptoms, risk and children . . . 10

Week 7 Psychometrics in practice . . . 11

Week 8 Diagnosis I: structure, use and limits . . . 11

Week 9 Differential diagnosis and comorbidity . . . 12

Week 10 Medical, neurological and substance rule-outs . . . 13

Week 11 Observed assessment interview . . . 13

Week 12 Relational and systemic diagnosis . . . 13

Week 13 Cultural formulation and bias . . . 15

Week 14 Feedback and report writing . . . 15

Week 15 Integration . . . 16

Glossary . . . 16

Self-assessment . . . 16

- Multiple choice . . . 16
- Answer key . . . 17
- Short answer . . . 17
- Essay (choose one, 2,000 words) . . . 17

Further reading . . . 17

Course leader's summary. You will learn to gather multi-informant data, use standardised measures properly, formulate relationally, use diagnosis without being used by it, and communicate all of it to a family in words they can use. The course culminates in a full assessment report of publishable quality.

How to use this book

Assessment is the phase of therapy where the most harm and the most help can be done. Work through the weeks in order: the sequence matters, because each week's skill depends on the previous one. The final assignment is a complete assessment report the same document type you will write for courts, schools and psychiatrists for the rest of your career.

1. Course description

Family assessment answers four questions: What is happening? How does it work? What maintains it? What would help? Diagnosis answers a fifth: Does this presentation meet criteria for a recognised disorder? Both matter, and they are not the same activity.

The course teaches the **multi-informant, multi-method** standard: self-report, collateral report, observation of interaction, standardised measures, records, and the family's own account integrated in a formulation that is shared, tested and revised.

2. Learning outcomes

#	Outcome	Assessed
LO1	Plan and conduct a multi-informant family assessment	Observed assessment
LO2	Administer, score and interpret at least four standardised measures	Report
LO3	Use DSM-5/ICD-11 criteria appropriately, including their limits	Exam
LO4	Produce a 5-P formulation with relational hypotheses and a risk screen	Report
LO5	Identify medical, substance-related and cultural rule-outs	Exam
LO6	Give assessment feedback to a family in accessible language	Skills assessment
LO7	Write a report that is accurate, defensible and free of jargon	Report

3. Assessment

Component	Weight	Due
Measure administration and scoring (portfolio)	20%	Week 7
Observed assessment interview (30 min)	25%	Week 11

Component	Weight	Due
Full assessment report (3,000 words + appendices)	40%	Week 15
Examination	15%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Purposes of assessment: clinical, legal, institutional	Map the purposes in a case
2	The multi-informant principle; who to see and in what order	Design an assessment plan
3	History taking: development, family, medical, psychiatric, substance	Structured history interview
4	Interaction observation: what to look for and how to record	Live observation coding
5	Standardised measures I: relationship and family functioning	Score and interpret
6	Standardised measures II: symptoms, risk, children	Score and interpret
7	**Measure portfolio due**; psychometrics in practice	Peer review of portfolios
8	Diagnosis I: DSM-5 and ICD-11 structure, use, limits	Criteria exercise
9	Diagnosis II: differential diagnosis and comorbidity	Case differentials
10	Medical, neurological and substance rule-outs	Referral-writing lab
11	**Observed assessment interview**	Assessment and feedback
12	Relational and systemic diagnosis: beyond the individual	Formulation workshop
13	Cultural formulation and bias in assessment	Cultural formulation exercise
14	Feedback, report-writing and shared decision-making	Report clinic
15	Integration: assessment that changes treatment	Case presentations

Week 1 Purposes of assessment

Assessment is never neutral. Ask **who wants it and what they will do with it**.

Purpose	Who commissions	What they need
Clinical	The family	Understanding, direction, relief
Institutional	School, employer, agency	A defensible opinion and a recommendation
Legal	Court, solicitor, child protection	Facts, opinion within expertise, documentation
Research	Investigator	Standardised, comparable data
Self-understanding	The family	Narrative coherence

The trap of multiple purposes. A family may come for help and be sent by a court. The therapist must be explicit about which hat they are wearing, because the duties differ a clinical assessment prioritises the client's interests; a forensic assessment prioritises accuracy for the court and is not confidential in the same way.

> **PRACTICAL**
> Take any referred case. List every party with an interest in the assessment and what each wants. Identify where the interests conflict.

Week 2 The multi-informant principle

Rule. Never rely on one informant for a relational question. Mothers, fathers, children and teachers systematically disagree, and the disagreement is clinically informative.

A workable order of contact.

1. **Referrer** (5 10 minutes by phone): what is the concern, what has been tried, is there risk?
2. **Whole family** (60 90 minutes): joining, each person's view, interaction observation, contract.
3. **Subgroups** as indicated: parents alone, each child, each partner separately (always screen IPV separately).
4. **Collateral** with consent: school, GP, previous therapist.
5. **Measures** completed between sessions.
6. **Feedback session**: share the formulation, invite correction.

Who is missing? Fathers, paternal relatives, non-resident children, step-parents, grandparents who co-parent, and the person the family has decided not to discuss. Ask: "Who else is part of this family who is not here? Who would disagree with what has been said today?"

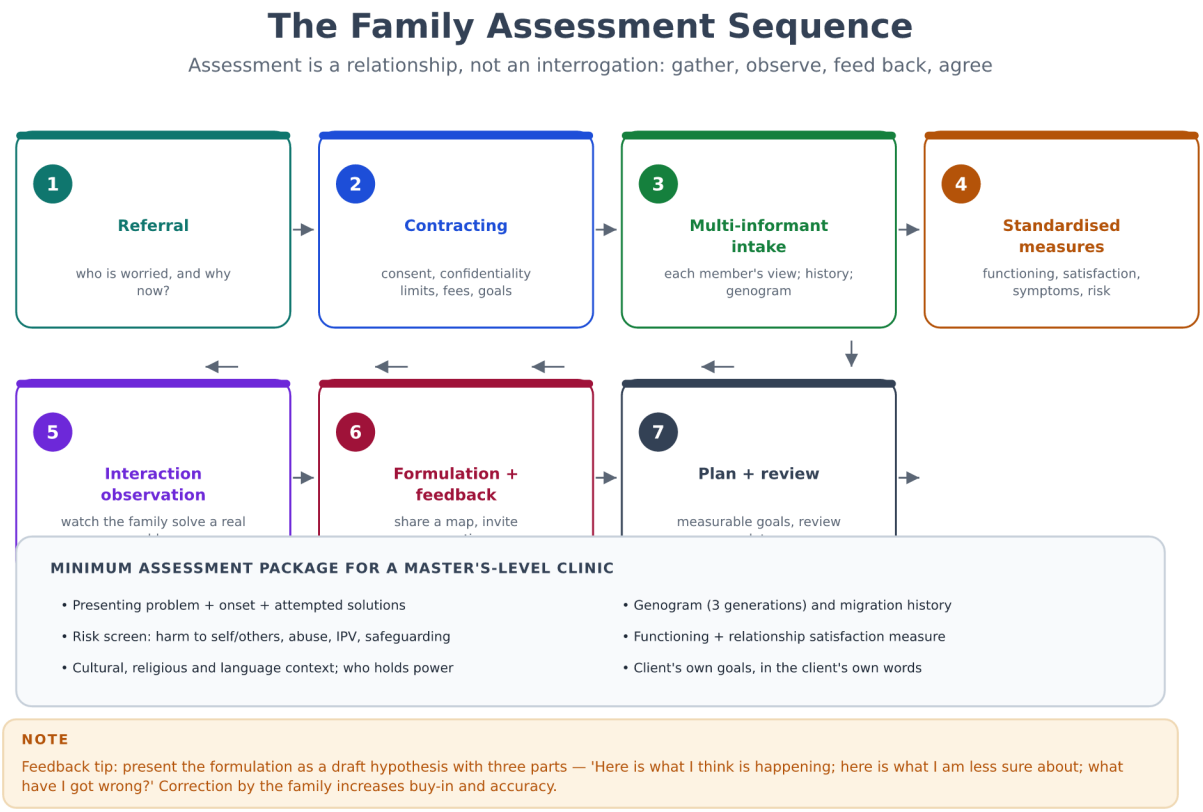


Figure 2.1 The assessment sequence from referral to review

> **PRACTICAL**
> For a case you know, design the assessment plan: who, in what order, how long, and why. Defend the order.

Week 3 History taking

A structured history prevents the two classic errors: never asking, and asking everything.

Domains (45 minutes is enough if you are organised).

Domain	Key questions
Presenting problem	Onset, course, severity, triggers, attempted solutions, previous treatment
Developmental	Pregnancy, birth, milestones, illnesses, separations, schooling
Family of origin	Genogram (3 generations), roles, losses, secrets, migration, violence, mental illness
Current family	Household composition, rules, routines, roles, money, division of labour, sexuality
Medical	Chronic illness, medication, sleep, head injury, neurological symptoms
Psychiatric	Previous diagnoses, hospitalisations, self-harm, suicide attempts, treatment response
Substance use	Alcohol, tobacco, cannabis, prescription, other; quantity, frequency, consequences

Domain	Key questions
Social	Work, school, faith community, friends, housing, legal issues, finances
Risk	Harm to self/others, abuse, neglect, IPV, safeguarding
Strengths	What has kept this family going? Who helps? What works?

The strengths column is not decoration. Families referred for assessment have usually been described in deficit terms for years. A careful strengths inventory changes the emotional tone of everything that follows and predicts engagement.

Week 4 Interaction observation

You do not have to rely on what families say about themselves; you can watch them.

Four standard tasks.

1. **The problem discussion (10 minutes).** "Talk to each other about the problem for ten minutes. I will not interrupt." Observe: who speaks, who is interrupted, who looks at whom, who is silent, how it ends.
2. **The planning task (5 minutes).** "Agree on one change you will make this week." Observe: negotiation, hierarchy, coalition, flexibility.
3. **The positive reminiscence (5 minutes).** "Tell each other about a good day you had together." Observe: affection, repair capacity, humour.
4. **The child-focused task.** "Decide together how to respond to the next tantrum." Observe: who decides, whether the child is consulted, whether the parents support each other.

What to record (behavioural, not judgemental).

Record	Not
"Father interrupted mother 7 times in 10 minutes; she stopped speaking after minute 6."	"Father was dominant."
"Daughter looked at mother before answering each question."	"Daughter is enmeshed."
"Parents agreed on the plan in 90 seconds; the child's objection was not addressed."	"Weak parenting."

The five-minute structural map. Draw the seating, the direction of speech, and the coalitions you observed. It will be more useful than a page of adjectives.

Week 5 Standardised measures I: relationship and family functioning

Why measure. To obtain a baseline, compare informants, track change, communicate with other professionals, and protect against your own impression management.

Core measures.

Measure	What it measures	Items	Notes
DAS-4 / RDAS	Couple satisfaction	4 / 14	Brief, widely used; cut-offs published
FACES-IV	Cohesion and flexibility	42	Psychometrics debated; use with caution
FAD (McMaster)	Family functioning	60	Six subscales; strong evidence base
Family APGAR	Perceived family support	5	Very brief; useful for screening
WAI-SR	Therapeutic alliance	12	Adapt for family members individually

Scoring discipline. Use the manual's scoring, not a photocopied table. Report raw and standardised scores, the norm group, and the cut-off used. State what a score does not mean: a high DAS score does not mean the couple is safe.

> **PRACTICAL**
> Score an RDAS from a completed (simulated) form: compute the total, compare with the published cut-off, and write three sentences you could say to the couple.

Week 6 Standardised measures II: symptoms, risk and children

Symptom measures. PHQ-9 (depression; note item 9 is a suicide item and must be followed up), GAD-7 (anxiety), AUDIT (alcohol), DUDIT (drugs), PC-PTSD-5 or PCL-5 (trauma), YSQ or DERS (emotion regulation) as indicated.

Child measures. SDQ (emotional and behavioural, 4 17, parent/teacher/self), RCADS (anxiety and depression), CBCL (broadband; longer), Conners (attention). Always obtain multiple informants and report them separately.

Risk measures. Columbia Suicide Severity Rating Scale (C-SSRS) or a structured clinical interview; do not substitute a questionnaire for a conversation.

Interpretation rules.

1. Measure the same construct with the same instrument at follow-up.
2. Report informant discrepancies rather than averaging them away.
3. Treat screening positives as questions, not diagnoses.
4. Consider literacy, language and cultural fit before interpreting a score.

> **PRACTICAL**
> Administer the SDQ to a volunteer parent and the self-report to a peer playing an adolescent. Compute both. Compare. Write what the discrepancy might mean.

Week 7 Psychometrics in practice



Figure 7.1 Validity and reliability: four targets, three methods

Four questions to ask of any instrument.

1. **What is the norm group?** If it was normed on US undergraduates, a Nigerian clinical sample's score needs careful interpretation.
2. **What is the internal consistency?** .70 is the usual minimum for research; .90 is desirable for individual clinical decisions.
3. **Has it been validated locally or in a similar population?** Search before you use.
4. **Is it responsive to change?** A measure that cannot detect improvement is useless for outcome monitoring.

Measurement invariance. The technical question "does this instrument measure the same thing across groups?" is clinically urgent. If a scale behaves differently for men and women, or across languages, then a difference in scores may reflect the instrument, not the people.

Week 8 Diagnosis I: structure, use and limits

What DSM-5 and ICD-11 are. Classification systems that group patterns of signs and symptoms into categories, using criteria, thresholds and durations. They are reliable vocabularies with imperfect validity: they describe clusters, not known disease entities.

The five DSM-5 axes are gone (they were removed in DSM-5, replaced by a single axial diagnosis plus separate notation of psychosocial stressors, disability and contextual factors). Know the current structure even if you trained on the old one.

Using diagnosis well.

1. Diagnose to communicate, not to explain. "Generalised anxiety disorder" tells a colleague what to expect; it does not tell you why this woman, in this family, at this time.
2. Diagnose only what you can defend with criteria.
3. Record uncertainty explicitly: "provisional", "rule out", "not otherwise specified".
4. Consider the function of a diagnosis in the family: it can unlock care, or become an identity, or a weapon in a custody dispute.

Using diagnosis badly.

- **Premature closure:** the first label constrains all subsequent observation.
- **Diagnostic overshadowing:** the label hides a physical illness, a trauma, or a relational problem.
- **Category fallacy:** assuming that because a category exists, a distinct disease exists.
- **Cultural misreading:** interpreting culturally normative behaviour (spiritual experience, deference, grief expression) as pathology.

> **PRACTICAL**
> Take one case and write two formulations: one purely diagnostic, one purely systemic. Then write the paragraph that integrates them.

Week 9 Differential diagnosis and comorbidity

The differential habit. For any presenting complaint, generate at least three alternatives before committing. Example: an adolescent's withdrawal.

Possibility	What would support it	What would rule it out
Major depression	Pervasive anhedonia, sleep/appetite change, 2+ weeks	No change in function; mood reactive
Social anxiety	Fear of scrutiny; avoidance specific to social settings	Comfortable with close friends
Trauma response	Intrusion, avoidance, hyperarousal after an event	No identifiable event
Substance use	Changed peer group, money, paraphernalia	Denial plus negative screen
Family crisis	Onset coincides with parental separation	No temporal link
Medical (e.g. anaemia, thyroid, sleep apnoea)	Fatigue, physical signs	Normal investigations

Comorbidity is the rule, not the exception. Depression and anxiety co-occur; substance use and trauma co-occur; child behaviour problems and parental mental illness co-occur. Prioritise: treat what is most dangerous, most disabling, or most treatable first, and treat the family context alongside.

Week 10 Medical, neurological and substance rule-outs

Refer, do not diagnose. Your job is to notice and to route.

Presentation	Rule out	Route
First episode of psychosis	Organic causes, substance-induced	Urgent psychiatric referral
Sudden confusion in an elder	Delirium, infection, medication	Same-day medical review
Fatigue, weight change, low mood	Anaemia, thyroid, diabetes, sleep apnoea	GP with specific request
Memory decline	Dementia vs depression vs delirium	Cognitive assessment service
Headaches, seizures, personality change	Neurological	Neurology
Erectile dysfunction, low libido	Vascular, hormonal, medication	Medical review before psychologising
Unexplained somatic complaints	Physical illness, somatisation	GP plus psychiatric liaison

Write a referral that works. State what you observed, what you want excluded, why now, and who to contact. "Please assess and advise" produces a delay; "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue and low mood, as I am considering a diagnosis of depression and would like to rule out physical causes before starting therapy" produces a response.

Week 11 Observed assessment interview

Thirty minutes, assessed: contract and confidentiality, each person's view, interaction observation task, risk screen, strengths, and a closing summary with next steps. The family rates their experience of being assessed on three dimensions: heard, not judged, clear about what happens next.

Week 12 Relational and systemic diagnosis

Individual diagnosis does not describe a relationship. Three ways of adding the relational layer:

1. Relational descriptors. Not a code, but a description: "a couple in a pursue withdraw cycle with low repair capacity"; "a family with a cross-generational coalition and a disengaged father"; "a family at a stage transition with no renegotiation".

2. Global functioning of the relationship. Instruments such as the GARF (Global Assessment of Relational Functioning) rate the family or couple's functioning along a continuum, with anchors. Useful for outcome monitoring and for communicating severity.

3. The formulation as the relational diagnosis. The 5-P grid plus relational columns is the most clinically useful "diagnosis" a family therapist produces.

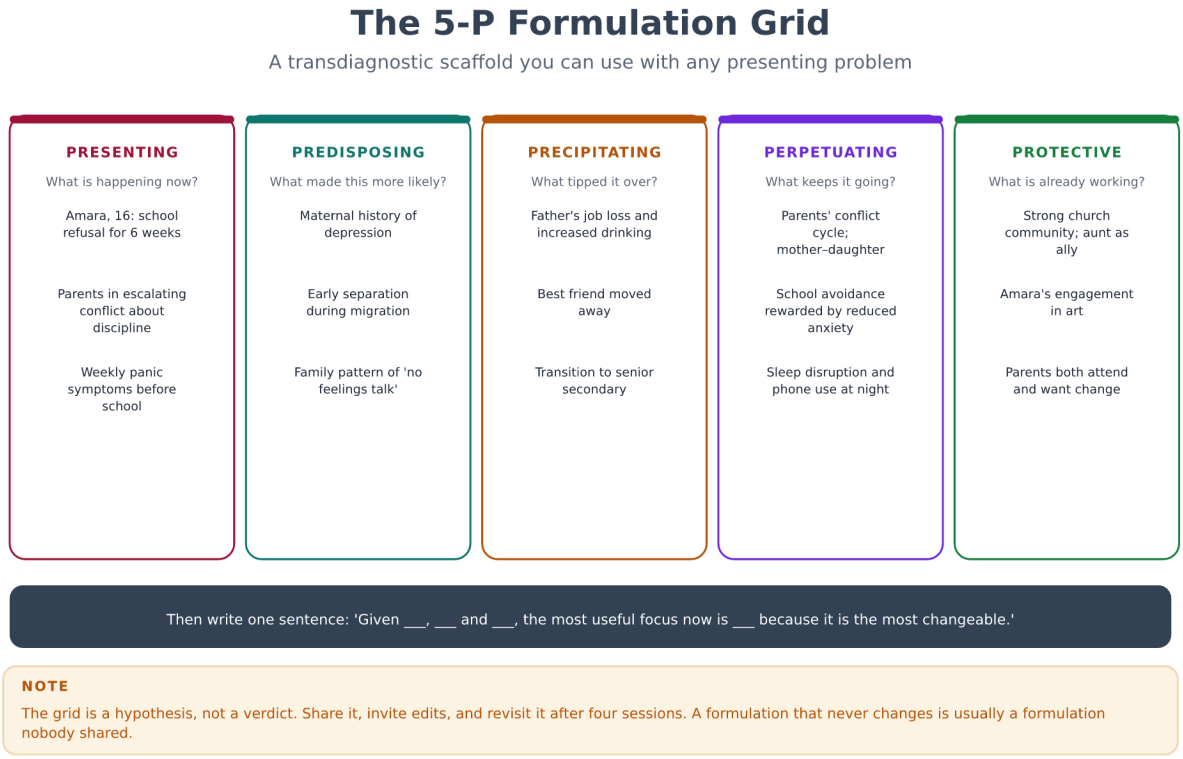


Figure 12.1 The 5-P formulation grid applied to one family

Worked formulation (Adeyemi family, condensed).

> **PRESENTING.** Amara, 16, has not attended school for six weeks; panic symptoms on school mornings; escalating parental arguments about discipline. **Predisposing.** Maternal history of depression; paternal alcohol misuse; migration removed the extended family's daily support; a family rule that feelings are not discussed. **Precipitating.** Father's redundancy, a best friend's relocation, and the transition to senior secondary school, all within four months. **Perpetuating.** A pursue withdraw cycle between the parents; a mother daughter coalition that stabilises the marital distance; the relief Amara experiences when she stays home, which reinforces avoidance; disrupted sleep and night-time phone use. **Protective.** Both parents attend; a trusted aunt; church community; Amara's engagement with drawing; the family's capacity for humour. **Hypothesis (testable).** If the parents can complete one conflict conversation without Amara's involvement, her morning panic will reduce within three weeks. **Focus:** de-escalate the couple cycle while building a graded return-to-school plan that does not depend on Amara's compliance alone.

Week 13 Cultural formulation and bias

The Cultural Formulation Interview (CFI) a set of 16 questions from DSM-5 gives structure to what many clinicians do informally:

1. What brings you here, in your own words?
2. What do you call this problem?
3. What do you think caused it?
4. How severe is it, and what worries you most?
5. What has helped, and what has not?
6. What kind of treatment do you expect?
7. Is there anything about your background or identity I should know?

Sources of bias in assessment.

Bias	Effect	Mitigation
Availability	Over-diagnosing what you saw last week	Use structured criteria
Confirmation	Seeking evidence for your first hypothesis	Actively seek disconfirmation
Cultural	Pathologising difference	Ask about meaning; consult
Gender	Reading assertion in women as pathology; in men as strength	Check your language
Class	Reading material hardship as neglect	Ask about money explicitly
Attribution	Blaming parents for adolescent behaviour	Include context and strengths

---

Week 14 Feedback and report writing

Feedback is an intervention. Structure it in four parts: what I heard, what I think is happening, what I am unsure about, what I suggest. Then stop talking and invite correction.

The report. Write for three readers: the family, the referring professional, and a future clinician who has never met you. Suggested structure:

1. Identifying information (minimal no more than necessary)
2. Reason for referral (in the referrer's words)
3. Consent, confidentiality and who was seen
4. Sources of information
5. Presenting concerns, from each informant
6. History (developmental, family, medical, psychiatric, substance, social)
7. Observation of interaction
8. Measures administered, with scores and interpretation
9. Risk assessment and outcome

10. Formulation (5-P plus relational hypothesis)
11. Diagnosis, if any, with criteria met and differentials considered
12. Recommendations, with who will do what
13. Review date and signature

Style rules. No adjectives about people; no speculation without labelling; no jargon the family cannot read; no more than two pages for the family copy.

> **PRACTICAL**
> Write the family-facing summary (one page, no jargon) of your full report, and test it on a non-clinician.

Week 15 Integration

Assessment is the moment a family learns what kind of problem they have. Get it right and therapy has a spine; get it wrong and every subsequent session is spent working on the wrong thing. The standard is not comprehensiveness; it is **sufficiency** enough information to make a defensible formulation and a workable plan, gathered in a way that leaves the family feeling respected.

Glossary

Collateral information data from third parties with consent. **Cultural formulation** a structured account of the cultural context of a presentation. **Diagnostic overshadowing** attributing a physical or relational problem to a psychiatric label. **Differential diagnosis** the systematic consideration of alternative explanations. **GARF** Global Assessment of Relational Functioning. **Measurement invariance** whether an instrument behaves equivalently across groups. **Multi-informant assessment** gathering data from several people who see the person in different contexts. **Rule-out** a condition that must be excluded before a diagnosis is confirmed. **Screening** a brief procedure identifying those who need fuller assessment.

Self-assessment

Multiple choice

1. The multi-informant principle exists because: (a) families lie (b) informants see different contexts and disagreement is informative (c) it is legally required (d) it fills the session

2. A high couple-satisfaction score means: (a) the relationship is safe (b) the relationship is satisfactory to the respondent, not that it is safe (c) the couple will stay together (d) no further assessment is needed

3. Diagnostic overshadowing refers to: (a) over-diagnosing (b) a label hiding another problem (c) comorbid diagnoses (d) cultural bias

4. The interaction task "agree on one change this week" is used to observe: (a) memory (b) negotiation, hierarchy and coalition (c) IQ (d) attachment classification

5. Which is the best referral request? (a) "Please assess and advise" (b) "Please see this patient" (c) "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue" (d) "Please confirm my diagnosis"

6. The 5-P formulation stands for presenting, predisposing, precipitating, perpetuating and: (a) psychiatric (b) protective (c) prognostic (d) provisional

7. Screening instruments should be treated as: (a) diagnoses (b) questions requiring follow-up (c) evidence of malingering (d) definitive cut-offs

8. The CFI is: (a) a personality test (b) a cultural formulation interview (c) a child functioning index (d) a court report

9. The strengths inventory matters because it: (a) is required by codes (b) changes tone and predicts engagement (c) replaces diagnosis (d) shortens assessment

10. A treating therapist should generally decline to conduct: (a) an intake (b) a custody evaluation of their own client (c) a genogram (d) an outcome measure

Answer key

1 b 2 b 3 b 4 b 5 c 6 b 7 b 8 b 9 b 10 b

Short answer

1. List the five parties who may have an interest in an assessment and what each wants.
2. Give three behavioural (not judgemental) observation statements.
3. What are the four parts of feedback to a family?
4. Name three biases in assessment and one mitigation for each.
5. Why must IPV screening be conducted separately from the partner?

Essay (choose one, 2,000 words)

1. "Diagnosis in family therapy is both necessary and dangerous." Discuss.
2. Using a disguised case, present a full assessment and defend your formulation against two alternatives.
3. Critically evaluate the claim that standardised family measures are culturally biased.

Further reading

- American Psychiatric Association. (2022). DSM-5-TR. APA Publishing.
- World Health Organization. (2019/2022). ICD-11.

- Carr, A. (2019). Family Therapy: Concepts, Process and Practice (3rd ed.). Wiley.
- Snyder, D. K., Heyman, R. E., & Haynes, S. N. (2005). Evidence-based approaches to assessing couple distress. Psychological Assessment, 17(3), 288 307.
- Sperry, L. (2012). Family Assessment: Contemporary and Cutting-Edge Strategies. Routledge.

Next: MFT 708 Cultural Competence in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 8 OF 28

MFT 708

Cultural Competence in Therapy

14 teaching weeks 5 practicals 2 worked cases 2 figures

SEMESTER ONE

1.71 units Prerequisite: MFT 701

> Culture is not a topic added to therapy; it is the medium in which therapy happens.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 708 Cultural Competence in Therapy

MSc Marriage and Family Therapy Semester One 1.71 Units 14 teaching weeks Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 708 Cultural Competence in Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	**Cultural Competence in Therapy**	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 What culture is and is not . . . 6

Week 2 Competence, humility, safety . . . 7

Week 3 The ADDRESSING framework . . . 7

Week 4 Race, racism and the clinical encounter . . . 9

Week 5 Ethnicity, tribe, language and migration . . . 9

Week 6 Religion and spirituality as clinical structure . . . 10

Week 7 Class, poverty and material reality . . . 11

Week 8 Gender, power and the politics of the family . . . 11

Week 9 Cultural assessment observed interview . . . 12

Week 10 Sexuality, gender identity and family acceptance . . . 12

Week 11 Disability, chronic illness and care . . . 13

Week 12 Polygyny, extended family and communal child-rearing . . . 13

Week 13 Decolonising therapy . . . 14

Week 14 Integration: your cultural position statement . . . 15

Glossary . . . 16

Self-assessment . . . 16

- Multiple choice . . . 16
- Answer key . . . 17
- Short answer . . . 17
- Essay (choose one, 2,000 words) . . . 17

Further reading . . . 17

Course leader's summary. Culture is not a topic added to therapy; it is the medium in which therapy happens. This course moves you from "knowing about cultures" to working skilfully with difference, power and meaning including your own.

How to use this book

You will be uncomfortable at points in this course. That is not a design flaw; it is the learning. Each week pairs a **concept** with a **practice** that requires you to examine your own position. Do the exercises honestly and keep them private this is formation, not performance.

1. Course description

The field's language has shifted from cultural competence (a set of knowledge and skills, often taught as a list of group traits) to cultural humility (a lifelong process of self-examination, redress of power imbalance, and institutional accountability) and cultural safety (the outcome judged by the person receiving care).

This course teaches all three, and is honest about their limits. It covers race and racism, ethnicity, religion and spirituality, class and poverty, gender and power, sexuality, disability, migration and displacement, language and interpreting, and the intersection of all of these in a single family.

2. Learning outcomes

#	Outcome	Assessed
LO1	Distinguish cultural competence, humility and safety, and use each appropriately	Exam
LO2	Conduct a cultural assessment using a structured framework	Observed interview
LO3	Analyse power, privilege and intersectionality in a clinical relationship	Reflective portfolio
LO4	Work effectively with interpreters and in a second language	Skills assessment
LO5	Adapt assessment and intervention to extended-family, faith-based and polygynous structures	Case analysis
LO6	Recognise and interrupt racism, ethnocentrism and bias in your own practice	Portfolio, viva
LO7	Critically appraise the cultural assumptions of the major therapy models	Essay

3. Assessment

Component	Weight	Due
Reflexive portfolio (6 entries 400 words)	30%	Fortnightly
Cultural assessment observed interview (25 min)	25%	Week 9
Case analysis with cultural formulation (2,500 words)	45%	Week 14

4. The fourteen-week plan

Week	Topic	Practice
1	What culture is and is not	Cultural genogram (own family)
2	Competence, humility, safety: history and critique	Concept comparison
3	The ADDRESSING framework	Structured cultural interview
4	Race, racism and the clinical encounter	Microaggression analysis
5	Ethnicity, tribe, language and migration	Interpreter role-play
6	Religion and spirituality as clinical structure	Faith-history interview
7	Class, poverty and material reality	Budget exercise
8	Gender, power and the politics of the family	Power mapping
9	**Cultural assessment observed interview**	Assessed
10	Sexuality, gender identity and family acceptance	Coming-out family session
11	Disability, chronic illness and care	Accessibility audit
12	Polygyny, extended family and communal child-rearing	Structural adaptation
13	Decolonising therapy: models, manuals and evidence	Critical appraisal
14	Integration: your cultural position statement	Viva

Week 1 What culture is and is not

Culture is the shared, learned system of meanings, practices, values and identities through which a group makes sense of the world. It is dynamic, internalised, contested and partially unconscious.

Five things culture is not.

1. **Not nationality.** "Nigerian culture" contains hundreds of languages and dozens of marriage systems.
2. **Not a personality.** Culture sets a range of possible behaviours, not a script every member follows.
3. **Not static.** Families migrate, convert, intermarry and change class; their culture changes with them.

4. **Not only "the other".** You have a culture too, and so does your profession.
5. **Not an excuse.** "That is our culture" can describe a practice or justify coercion; the therapist must be able to hold respect and moral clarity at the same time particularly about violence.

The clinical implication. Never ask "what do people from X do?" Ask "what does this family do, what does it mean to them, who in the family disagrees, and how does it interact with the problem?"

> **PRACTICAL**
> Draw your own cultural genogram: languages, religions, migrations, class position, ethnic identities, and the rules you absorbed about marriage, money, gender and feelings. Note what you would find difficult to say to a family that did the opposite.

Week 2 Competence, humility, safety

Concept	Core claim	Strength	Limitation
Cultural competence	Knowledge and skills about groups can be taught	Practical, trainable	Risks stereotyping; assumes culture is a trait
Cultural humility	Lifelong self-reflection, redress of power, institutional accountability	Attends to power and to the self	Can become inward-looking; hard to assess
Cultural safety (Ramsden)	The recipient judges whether care is safe	Centres the client's experience	Requires institutional change, not only individual skill

The synthesis used in this programme. Humility as the stance, competence as the skills, safety as the criterion. You reflect on your position (humility), use structured tools and knowledge (competence), and ask the family whether the encounter was safe (safety).

Why "I treat everyone the same" is not an ethical position. Treating everyone the same preserves the advantages of the majority, because the default is not neutral it was built for someone.

Week 3 The ADDRESSING framework

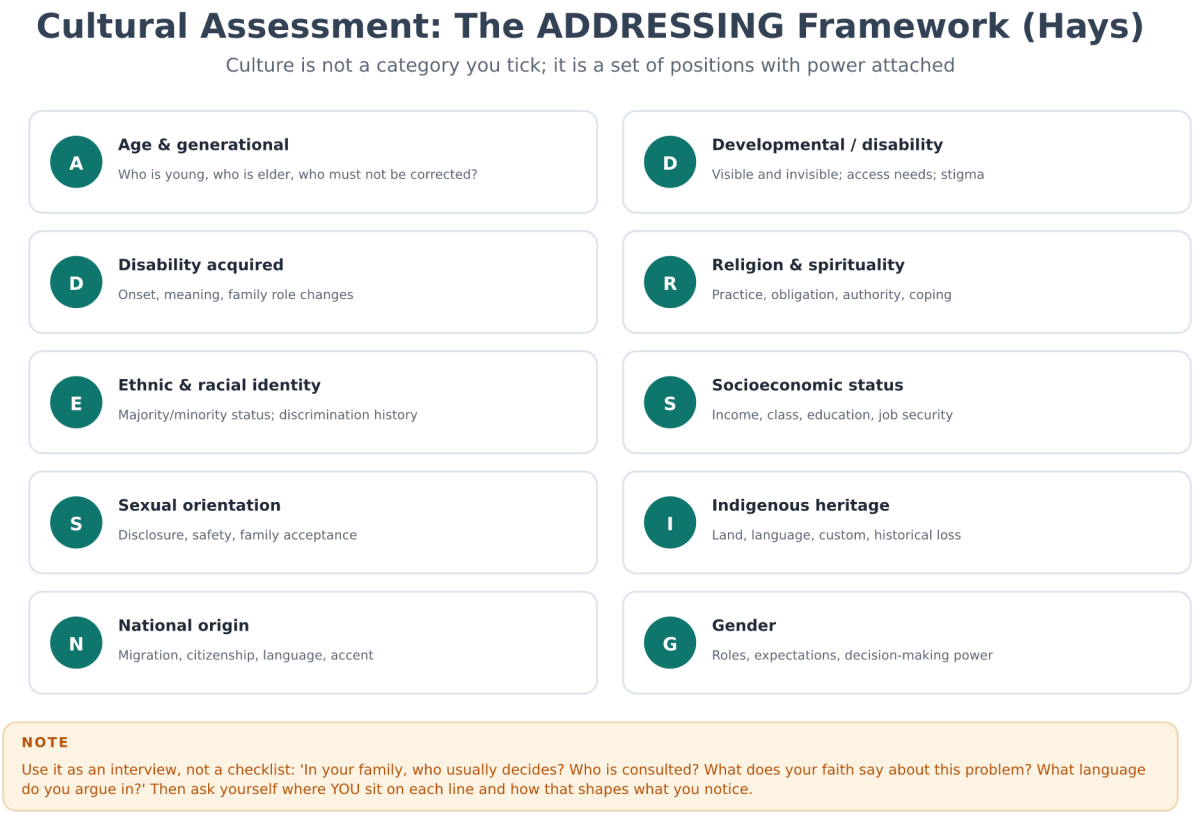


Figure 3.1 ADDRESSING: ten cultural positions with power attached

Hays's framework asks you to assess ten dimensions, for the family **and for yourself**:

Age and generation Developmental disability Disability acquired later Religion and spirituality Ethnic and racial identity Socioeconomic status Sexual orientation Indigenous heritage National origin Gender.

Interview prompts that work.

- "In your family, who usually decides? Who is consulted? Who is told afterwards?"
- "What does your faith say about this problem? Who in your religious community knows?"
- "Which language do you argue in? Which language do you pray in?"
- "When you were growing up, was money talked about?"
- "What did your family teach you about people from [another group]?"

The power question. For each dimension, ask: is this person in the majority or the minority in this society, in this clinic, and in this room? The same person may be privileged on one axis and marginalised on another this is intersectionality.

> **PRACTICAL**
> Conduct a 15-minute structured cultural interview with a peer using five ADDRESSING domains. Then reverse roles. Debrief: which question was hardest to ask, and why?

Week 4 Race, racism and the clinical encounter

Definitions. Race is a social category with no biological basis; its effects are nonetheless real and measurable. Racism is the system of advantage and disadvantage organised around race, operating at interpersonal, institutional and internalised levels.

Three clinical implications.

1. **Racism is a chronic stressor with health consequences** including hypertension, depression, and adverse birth outcomes. It belongs in the assessment, not in a footnote.
2. **Therapy is not a racism-free space.** Families may test you, protect you, or perform for you. Ask: "Have you had experiences in health settings where you were treated badly because of your background?"
3. **Microaggressions occur in clinical settings** comments, assumptions, lowered expectations. They are often unintentional and are still harmful.

Four common therapist failures.

Failure	Example	Repair
Colour-blindness	"I don't see race"	Acknowledge difference; ask about its meaning here
Defensiveness	"I'm not racist, but "	Focus on impact, not intent
Over-attribution	Explaining everything by race	Differentiate race from other factors
Exoticising	Treating the family as a cultural exhibit	Ask about ordinary life too

Talking about race when it has not been mentioned. Wait for a cue, then ask directly: "I wondered whether the fact that I am [X] and you are [Y] matters here. Sometimes it does, sometimes it doesn't." Offering the question is often more important than the answer.

> **PRACTICAL**
> Analyse three microaggression scenarios from a clinical setting: what was said, what was meant, what was heard, what a repair would sound like.

Week 5 Ethnicity, tribe, language and migration

Ethnicity and tribe. In Nigeria and across Africa, ethnic identity carries histories of majority/minority relations, language, land, and in some cases conflict. Ask about it as a relationship question: how do the two families regard each other's ethnicity? Was the marriage contested? Whose language do the children speak?

Migration. Migration is a family event, not an individual one. Key clinical themes:

- **Split households** one parent abroad, children with grandparents, "sent" children.

- **Role reversal** children who acquire the new language become the family's intermediaries.
- **Loss of the extended-family buffer** the practical support that made parenting possible.
- **Status loss** a professional who becomes a cleaner; a respected elder who becomes invisible.
- **Legal precarity** the constant threat of removal, which changes every decision.

Language. A family may think in one language about emotion and another about work. Ask: "In which language do you feel most like yourself? Which language do you dream in?" Offer sessions in the family's preferred language where you can; where you cannot, use an interpreter properly.

Working with interpreters the standard.

1. Brief before: purpose, confidentiality, first-person interpretation, do not summarise or advise.
2. Seat the interpreter beside you, facing the family.
3. Speak to the family, not to the interpreter ("How do you feel?" not "Ask her how she feels").
4. Allow longer for interpretation; do not fill pauses.
5. Debrief afterwards: what was difficult, what was not said, any cultural information.
6. **Never** use a child as interpreter for anything clinical.

> **PRACTICAL**
> Interpreter role-play: one student interprets, one conducts, one plays a family member who switches languages. Practise briefing, first-person interpretation, and debriefing.

Week 6 Religion and spirituality as clinical structure

In most of the world, faith is not an "aspect" of family life; it is the organising framework. Asking about it is not optional.

What to ask. What is your faith? How does it shape daily life (prayer, diet, dress, holy days)? What does it say about marriage, divorce, sex, mental illness, healing, and death? Who is your religious authority? Have you consulted them about this problem? What did they say?

Three clinical patterns.

1. **Faith as resource.** Community, meaning, ritual, forgiveness, hope, practical support.
2. **Faith as stressor.** Guilt, prohibition of divorce, deliverance beliefs that attribute mental illness to sin, conflict between religious and medical models.
3. **Faith as site of conflict.** A couple from different traditions; conversion; a teenager leaving the faith; a pastor's authority competing with the therapist's.

Coordination, not competition. With consent, a conversation with the family's pastor, imam or elder can be more powerful than any technique. The aim is a shared plan, not a theological debate. You do not have to share the family's beliefs to take them seriously; you do have to be honest about your own.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent. She will not go to the police; the pastor has counselled her to be patient. The therapist must respect her faith and autonomy, hold the pastor as an ally where possible, and build a safety plan that does not depend on the husband's repentance. (See MFT 712, 724.)

Week 7 Class, poverty and material reality

The clinical error. Reading the effects of poverty as individual or family pathology: "inconsistent parenting" when the mother works three jobs; "neglect" when the family has no electricity; "disorganised" when the household has eight people in two rooms.

What to ask, without humiliating.

- "Is money tight at the moment? How tight?"
- "What happens at the end of the month?"
- "Who else depends on your income?"
- "Have you ever had to choose between food and a bill?"

Practical action. Have a short list of referral routes: food support, school fee assistance, legal aid, social welfare, health insurance. A therapist who ignores material reality becomes part of the family's helplessness.

The class position of therapy itself. Sessions cost money, require transport and time off, and are conducted in a register many families do not use. Adapt: flexible times, community venues, shorter paperwork, plain language, a written summary the family can keep.

> **PRACTICAL**
> Complete a household budget for a family with two minimum-wage incomes and two children. Then re-read a "poor parenting" case with that budget in mind.

Week 8 Gender, power and the politics of the family

Power is not the same as influence. Ask: who decides, who is consulted, who is informed, who is financially dependent, who can leave, who would be believed if they spoke?

Gender patterns you will meet.

Pattern	Presentation	Clinical risk if ignored
Traditional division of labour	Mother "does everything" and is exhausted; father "does nothing"	Therapist aligns with mother; father withdraws further
Head-of-household ideology	Father decides; mother adapts	Mother's distress is invisible; children learn silence
Female economic power	Wife out-earns husband	Status threat becomes the real argument
Male emotional restriction	He cannot say what he feels	Therapist misreads flatness as lack of love

The feminist critique that changed the field (Hare-Mustin, Goldner, Walters): early family therapy described "circular" patterns as though both parties had equal power. In violence, in economic dependence, and in cultural constraints, they do not. A circular description that ignores power becomes a sophisticated form of blame-the-victim.

Clinical practice. Name the asymmetry without moralising; ask the family whether they want it to be this way; work with the person who holds more power to use it differently; never insist a woman "communicate better" as a substitute for a man changing his behaviour.

Week 9 Cultural assessment observed interview

Twenty-five minutes, assessed against six criteria: use of a structured framework, curiosity rather than assumption, power-attentiveness, no stereotyping, integration of culture with the presenting problem, and a respectful close.

Week 10 Sexuality, gender identity and family acceptance

The clinical reality. In many settings, disclosure is unsafe, and families are torn between religious conviction, social pressure, and love for their child. Your task is not to resolve that for them but to make the conversation possible and to keep everyone safe.

Principles.

1. **Do not assume or ask for labels prematurely.** Follow the client's language.
2. **Assess safety first.** Is it safe at home, at school, at church? Who knows? What is the risk of disclosure?
3. **Work with the family's ambivalence**, not against it. "You have both said you love your son. You have also said you cannot accept this. I want to understand both, without asking you to give either up today."
4. **Never promise a family you will change the child**, and never promise a child you will change the family. Be honest with both.

5. **Know your legal context and your local referral routes**, including safe medical and mental health services.

When the family rejects. Rejection is a process with stages: shock, denial, negotiation, and in some families accommodation. Your job is to keep the door open, reduce harm, and protect the most vulnerable member.

Week 11 Disability, chronic illness and care

Two models. The medical model locates disability in the body; the social model locates it in barriers physical, attitudinal, institutional. Both matter clinically.

Assessment questions. What is the diagnosis and functional impact? Who provides care, and at what cost? What has the family had to give up? What is the meaning of the condition in this family's belief system (punishment, test, curse, genetics)? What accommodations are needed?

Avoiding two errors. Minimising ("it's not that bad") and catastrophising ("your life is over"). Ask the person directly: "What do you want me to understand about how this affects you?"

Sibling and parent impact. Caregiving burden falls unequally; siblings of disabled children often develop precocious competence and unspoken resentment. Name it as love and loss together.

Week 12 Polygyny, extended family and communal child-rearing

Do not treat structure as pathology. Household forms that differ from the Western nuclear norm are not automatically dysfunctional. Assess function, safety and fairness within the form.

Polygynous households assessment prompts.

- Who lives where, and who decides?
- How are resources and time allocated? Who keeps the accounts?
- What are the relationships between wives allies, rivals, or both?
- How do children understand their relationships to the other wives?
- Is the marriage legal under a statutory, customary or religious system (the answer affects rights)?
- Who in the household holds authority over the children?

Clinical themes that recur: jealousy and comparison, resource competition, co-wife coalitions and sub-coalitions, children as messengers, unequal treatment, and the husband's role as the only permitted channel of communication.

Communal child-rearing. In many families, a child is raised by a grandmother, an aunt, or a foster kin network. This can be protective (multiple attachment figures) or risky (unclear responsibility, unspoken injury). The question is not "who is the mother?" but "who is responsible for what, and does the child know who to go to?"

> **CASE**
> A husband proposes a second wife. The first wife presents with insomnia and anger; he presents with confusion. The therapist neither endorses nor condemns the practice. The work is: what does each understand the marriage to be? What does the first wife have a right to? What will the children be told? Are there religious and legal structures the family wants involved?

Week 13 Decolonising therapy

The critique. Most of the therapy models in this programme were developed in North America and Europe, tested on samples that were not representative, and then exported as universal. Three specific problems:

1. **Normative assumptions.** Individuation, self-expression and boundary-setting are treated as health. In many families, interdependence, duty and discretion are the markers of maturity.
2. **Evidence hierarchies.** Trials and manualised models carry more status than community knowledge and qualitative research, which subtly determines what can be funded and taught.
3. **Language.** "Enmeshment", "over-involvement", "dependence" are not neutral words; they encode a picture of the good family.

Responses that are more than rhetoric.

- Learn the local literature; cite African scholars; test instruments locally.
- Adapt models explicitly and say what you changed and why.
- Use the family's language and concepts in formulation.
- Include community and faith resources in treatment plans.
- Publish your adaptations so the field's evidence base changes.

And a caution. Decolonisation is not the replacement of one orthodoxy with another. Families are not monoliths, and "that is our culture" can silence a woman or a child as effectively as a Western manual can.

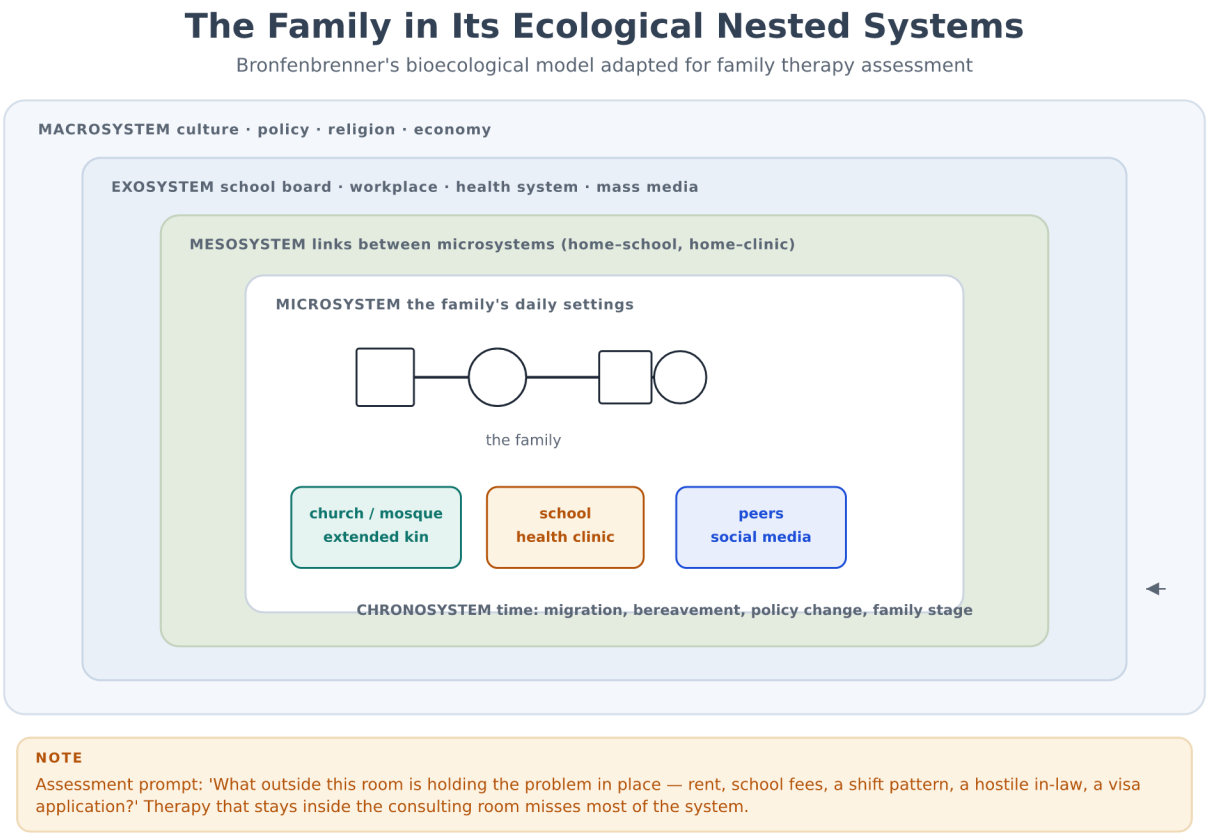


Figure 13.1 Ecological systems: the family inside institutions, culture and time

Week 14 Integration: your cultural position statement

Write 1,000 words: your cultural identities and their advantages and disadvantages; two families with whom you would struggle and why; the adaptations you will make; the assumptions of your preferred model that you will watch; and how you will know when you have got it wrong. Present it in a viva.

Signals that you have got it wrong (learn to recognise them): the family is polite and does not return; you are doing most of the talking; you have explained their culture to them; you feel defensive; you have used the word "they".

Glossary

Cultural humility a lifelong stance of self-examination and redress of power imbalance. **Cultural safety** the recipient's judgment that care is safe and free of assault on identity. **Cultural formulation** a structured account of culture in a presentation (e.g. the DSM-5 CFI). **Ethnocentrism** judging another culture by the standards of one's own. **Intersectionality** the interacting effects of multiple social positions. **Microaggression** a brief, commonplace indignity, often unintentional. **Power** the capacity to influence decisions, resources and meanings; distinct from influence. **Stereotype threat** the risk of confirming a negative stereotype about one's group. **Structural competency** the capacity to recognise and act on upstream social determinants.

Self-assessment

Multiple choice

1. "I treat everyone the same" is inadequate because: (a) it is impossible (b) the default is not neutral (c) clients dislike it (d) it is illegal

2. Cultural safety is judged by: (a) the therapist (b) the institution (c) the recipient of care (d) the code of ethics

3. Asking a family "what does your faith say about this problem?" is: (a) inappropriate (b) essential assessment (c) only for religious therapists (d) a legal requirement

4. An interpreter should: (a) summarise key points (b) interpret in the first person (c) advise the family (d) be the family's child

5. The social model of disability locates disability primarily in: (a) the body (b) barriers in society (c) genetics (d) family dynamics

6. A circular formulation that ignores power risks: (a) complexity (b) blaming the less powerful partner (c) over-diagnosis (d) cultural sensitivity

7. Intersectionality refers to: (a) cultural competence (b) interacting social positions (c) bilingualism (d) family structure

8. A family arrives with a co-wife arrangement. The therapist should first: (a) assess for pathology (b) endorse the arrangement (c) assess function, safety and fairness within the structure (d) refuse to work with them

9. Microaggressions in clinical settings are: (a) always intentional (b) often unintentional and still harmful (c) rare (d) not clinically relevant

10. Decolonising therapy means: (a) rejecting all Western models (b) adapting and testing models, citing local knowledge, changing the evidence base (c) using only local languages (d) avoiding diagnosis

Answer key

1 b 2 c 3 b 4 b 5 b 6 b 7 b 8 c 9 b 10 b

Short answer

1. Distinguish cultural competence, humility and safety in one sentence each.
2. Write four ADDRESSING prompts you would actually use.
3. What are the six steps of working with an interpreter?
4. Give three clinical effects of racism and one assessment question.
5. Name three culturally specific assumptions in EFT or structural therapy and how you would adapt them.

Essay (choose one, 2,000 words)

1. "Cultural humility is a stance, not a technique." Discuss with reference to two clinical encounters.
2. Critically examine the claim that family therapy models pathologise interdependence.
3. Analyse a case in which your own cultural position shaped the assessment, and what you would do differently.

Further reading

- Hays, P. A. (2016). Addressing Cultural Complexities in Practice (3rd ed.). APA.
- Tervalon, M., & Murray-Garc a, J. (1998). Cultural humility versus cultural competence. Journal of Health Care for the Poor and Underserved, 9(2), 117 125.
- McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.).
- Falicov, C. J. (2014). Latino Families in Therapy (2nd ed.). Guilford.
- Ramsden, I. (2002). Cultural Safety and Nursing Education. Victoria University of Wellington.
- Brady, M. (Ed.). (2012). The Oxford Handbook of the Philosophy of Culture. OUP.

End of Semester One. Next: MFT 709 Group Therapy Techniques

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 9 OF 28

MFT 709

Group Therapy Techniques

15 teaching weeks 5 practicals 1 figure

SEMESTER TWO

1.71 units Prerequisite: MFT 706

> Groups are the most efficient and the most unpredictable clinical format.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 709 Group Therapy Techniques

MSc Marriage and Family Therapy Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 709 Group Therapy Techniques. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	**Group Therapy Techniques**	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 Why groups work . . . 6

Week 2 Groups in family therapy practice . . . 7

Week 3 Design . . . 7

Week 4 Screening, consent and preparation . . . 8

Week 5 Yalom's therapeutic factors . . . 8

Week 6 Stages I: forming and storming . . . 10

Week 7 Stages II: norming, performing, adjourning . . . 10

Week 8 The here-and-now . . . 11

Week 9 Difficult members . . . 11

Week 10 Conflict, scapegoating and subgrouping . . . 12

Week 11 Co-facilitation . . . 12

Week 12 Observed co-facilitation . . . 13

Week 13 Couples and parents groups . . . 13

Week 14 Ethics and boundaries in groups . . . 13

Week 15 Evaluation and endings . . . 13

Glossary . . . 14

Self-assessment . . . 14

- Multiple choice . . . 14
- Answer key . . . 15
- Short answer . . . 15
- Essay (choose one, 1,800 words) . . . 15

Further reading . . . 15

Course leader's summary. Groups are the most efficient and the most unpredictable clinical format. This course teaches you to design a group, build a culture, work with the process rather than only the content, manage the difficult members, and co-facilitate without performing a marriage in front of the members.

How to use this book

A group cannot be learned from a book alone, but it can be ruined without one. Each week gives you the theory, a facilitation script, and a hazard. The practical sessions in this course run as a real group: you will be members before you are leaders, and you will write about both.

1. Course description

Group therapy is a distinct modality with its own theory of change: members help each other, and that mutual aid is the treatment. Family therapists use groups for couples (multiple couples in one group), parents, adolescents, men who have used violence, people in recovery, the bereaved, and children.

This course covers group design, composition, stages of development, therapeutic factors, process observation, difficult members, co-facilitation, ethics and evaluation. It is grounded in Yalom's interpersonal model and in systemic thinking about the group-as-a-system.

2. Learning outcomes

#	Outcome	Assessed
LO1	Design a group: purpose, membership, size, duration, setting	Group proposal
LO2	Explain and use Yalom's therapeutic factors	Exam
LO3	Identify and work with group process (here-and-now)	Observed facilitation
LO4	Manage difficult members and group crises safely	Viva, observed
LO5	Co-facilitate effectively, including managing disagreement	Peer and tutor rating
LO6	Screen and prepare members ethically	Screening role-play
LO7	Evaluate group outcomes	Evaluation plan

3. Assessment

Component	Weight	Due
Group design proposal (2,000 words)	25%	Week 6
Process reflection log (as member, 6 entries)	20%	Fortnightly

Component	Weight	Due
Observed co-facilitation (30 min)	30%	Weeks 12 14
Examination	25%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why groups work: mutual aid and the social microcosm	First group session (as member)
2	Types of group in family therapy	Observation of a live/recorded group
3	Design: aims, size, duration, open vs closed	Design clinic
4	Screening, consent and preparation	Screening role-play
5	Yalom's therapeutic factors	Factor-spotting exercise
6	**Group proposal due**; group stages I: forming and storming	Process observation
7	Group stages II: norming, performing, adjourning	Leader tasks by stage
8	The here-and-now: working with process	Process commentary drill
9	Difficult members: monopoliser, silent, hostile, help-rejecting	Structured rehearsals
10	Conflict, scapegoating and subgrouping in groups	Live intervention
11	Co-facilitation: division of labour, disagreement, repair	Co-therapy pairs
12	**Observed co-facilitation begins**	Assessed
13	Groups for couples and for parents	Design adaptation
14	Ethics, confidentiality and boundaries in groups	Incident analysis
15	Evaluation and endings	Outcome plan

Week 1 Why groups work

The core claim. In a group, members are simultaneously helpers and helped. This reciprocal process Yalom called it mutual aid produces learning that a one-to-one relationship cannot, because it is horizontal rather than vertical. The leader is not the only source of help; the membership is.

The social microcosm. A group is a small society in which each member eventually behaves as they do outside: the man who must control, the woman who apologises, the adolescent who deflects with humour. The group then becomes a laboratory in which those behaviours are noticed, felt, and changed in vivo.

Systemic framing. A group is a system: it has boundaries (who is in, who is out), roles, norms, a developmental history, and homeostatic mechanisms that resist change.

Everything you learned about families applies. The scapegoat in a group is playing the same structural role as the identified patient in a family.

> **PRACTICAL**
> Begin the experiential group this week. The group runs for the semester with two student co-leaders rotating. Write your first reflection on what it felt like to be a member.

Week 2 Groups in family therapy practice

Group type	Members	Typical aims
Couples group	4 6 couples	Communication skills, shared normalisation, vicarious learning
Parents group	6 10 parents	Behaviour management, support, reducing isolation
Adolescent group	6 8	Identity, peer feedback, social skills
Men's behaviour change	8 12	Accountability, gender norms, non-violence
Recovery group	8 12	Sobriety support, family reintegration
Bereavement group	6 10	Normalising grief, meaning-making
Psychoeducational	10 20	Information plus discussion
Multi-family group	4 6 families	Family family learning (strong evidence in psychosis and eating disorders)

Multi-family groups deserve particular attention in this programme: they combine systemic work with mutual aid, are cost-effective, and have good evidence in severe mental illness, eating disorders and adolescent conduct problems.

Week 3 Design

Eight design decisions. Each should be justified in your proposal.

1. **Aim** what change, for whom, measured how?
2. **Membership** who is included, who excluded, and on what criteria?
3. **Size** 6 10 for interactional groups; larger for psychoeducation.
4. **Duration and frequency** 8 16 sessions weekly is typical; state the rationale.
5. **Open or closed** closed builds cohesion; open tolerates attrition.
6. **Setting and materials** circular seating, privacy, accessibility.
7. **Leadership** solo or co-facilitated (co-facilitation is strongly preferred for beginners).
8. **Evaluation** pre/post measures, attendance, satisfaction, and a follow-up.

Composition traps. Mixing victims and perpetrators of violence in one group; mixing adolescents and adults; including two people in a current conflict (e.g. two co-wives, or a

couple in the same parents' group); including a member who knows another member socially without discussing it in advance.

> **PROPOSAL (25%)**
> A 2,000-word design document: population, aims, evidence base, membership criteria, screening protocol, session-by-session outline, facilitation plan, risk protocol, evaluation plan, budget.

Week 4 Screening, consent and preparation

Screening protects the group as much as the individual. Screen for: fit with the group's aims, current risk, ability to tolerate group feedback, concurrent individual therapy, and practical commitment.

The screening interview (30 minutes).

1. What brings you here, and what do you hope for?
2. What has helped before? What has not?
3. Have you been in a group before? How was it?
4. How do you usually handle conflict and criticism?
5. Is there anyone in the group you know? (Show the member list where appropriate.)
6. Are you currently safe? (Risk screen.)
7. Are you willing to keep confidentiality, knowing it cannot be guaranteed?

Consent must be specific. Explain that confidentiality is a group norm but not a guarantee; that members may be upset by others; that the leaders will break confidentiality only in the circumstances described; and that attendance and punctuality are part of the contract.

Preparation improves outcomes. Tell the member what to expect, what will be asked of them in the first session, and how to introduce themselves. Pre-group preparation reduces drop-out measurably.

> **PRACTICAL**
> Screening role-play with a volunteer who is ambivalent and has been pressured to attend. Practise: exploring ambivalence, explaining the frame, and declining a poor fit without rejecting the person.

Week 5 Yalom's therapeutic factors

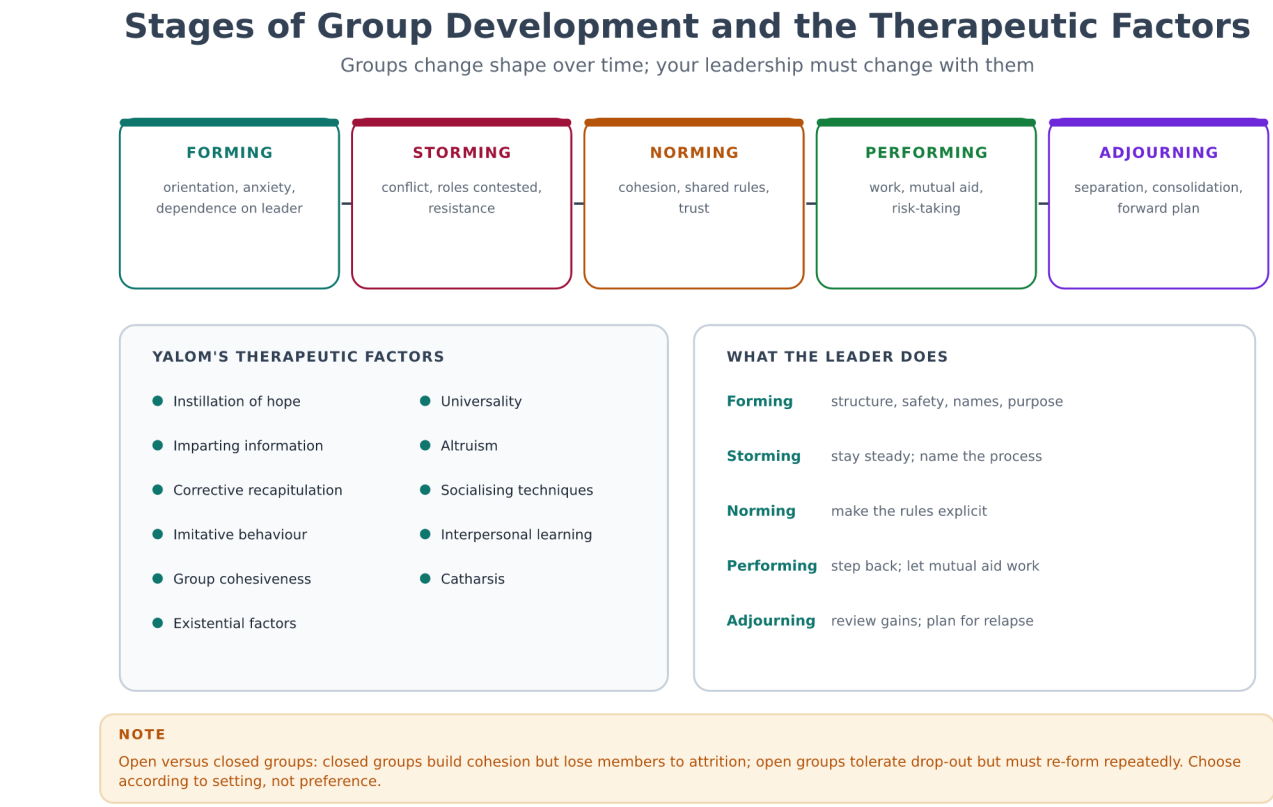


Figure 5.1 Group stages, therapeutic factors and the leader's changing task

Factor	What it looks like	How the leader strengthens it
Instillation of hope	A member describes recovery	Ask for specifics; keep the member connected to the group
Universality	"I thought I was the only one"	Name the shared experience explicitly
Imparting information	Advice, psychoeducation	Redirect advice-giving into dialogue
Altruism	A member helps another	Point out the help; let the helper be seen
Corrective recapitulation	Old family patterns reappear	Interpret the pattern in the here-and-now
Socialising techniques	Learning social feedback	Model and reinforce it
Imitative behaviour	Modelling the leader or a member	Use yourself deliberately
Interpersonal learning	Feedback changes behaviour	Make the group the site of the experiment
Group cohesiveness	Belonging, attendance, care	Protect the frame; address absences
Catharsis	Emotional release	Contain it; link it to meaning
Existential factors	Responsibility, mortality, isolation	Allow the conversation; do not rush to technique

The beginner's error: chasing catharsis. Tears without integration produce relief for a day and no change. The sequence is: **express understand experiment consolidate**.

Week 6 Stages I: forming and storming

Forming. Anxiety, politeness, dependence on the leader, search for structure. Members ask: "What are we supposed to do? Is this allowed?"

Leader task: provide structure and safety, model disclosure at an appropriate level, establish norms (punctuality, confidentiality, one person at a time, no advice without asking), and name the anxiety.

Script: "Most groups start quietly. Part of what we will do is notice what we are all avoiding."

Storming. Conflict, role competition, resistance, criticism of the leader, subgrouping. This is not failure; it is the group testing whether it can survive difference.

Leader task: stay steady, do not defend, name the process, and let the group negotiate. Avoid solving it for them.

Script: "There is something happening between the two of you that the rest of us are watching. Can we look at it together rather than settle it?"

> **PRACTICAL**
> Process observation. Two observers code the session for: who speaks, who is interrupted, who is silent, and three moments of process (as opposed to content). Feed back to the group in the last ten minutes.

Week 7 Stages II: norming, performing, adjourning

Norming. Cohesion, shared rules, mutual trust, humour. Attendance stabilises.

Leader task: consolidate the norms explicitly; use the cohesion to do deeper work.

Performing. The productive stage: mutual aid, confrontation with care, risk-taking, insight.

Leader task: step back. Your best sessions are the ones in which you spoke least and the group worked. Redirect all content to the members: "What does the group think?"

Adjourning. Ending, loss, consolidation, transfer to life outside.

Leader task: review gains, name the loss, have each member say what they gave and received, and make a forward plan. Do not let the group end by attrition.

Termination script: "We have four sessions left. I want us to use them for two things: what each of you has learned, and what each of you will do when the group is not here."

Week 8 The here-and-now

Two layers. Content is what the group is talking about; process is what is happening between the people while they talk about it. Groups change through process.

The four-step process commentary.

1. **Notice** something concrete: "Every time Ada speaks about her marriage, Femi looks at the clock."
2. **Name it to the group, not about the person:** "I notice something happening here."
3. **Ask the group to look:** "Does anyone else notice it? What do you think it is about?"
4. **Return it to the relationship:** "Femi, what is happening for you when Ada speaks?"

Timing rules. Do not process-comment in the first two sessions (no safety yet). Do not do it to demonstrate skill. Do it when the same pattern has occurred at least twice.

Hazards. Process commentary can feel like exposure. Use the group as the buffer ("does anyone else "), avoid naming individuals as specimens, and never interpret someone's process before you have checked it with them.

> **DRILL**
> Ten minutes of "process-only": the group may only speak about what is happening here, not about events outside the room. Debrief how uncomfortable it was.

Week 9 Difficult members

Member type	Behaviour	What it protects	Intervention
Monopoliser	Talks at length; fills silence	Anxiety, fear of invisibility	"I want to bring others in; you may find that hard." Set a turn-taking norm
Silent member	Says nothing; attends	Fear of exposure, cultural deference	Low-demand invitation; do not force; check in privately
Hostile/devaluing	Attacks the leader or a member	Shame, previous betrayal	Name impact; set the boundary; explore what it protects
Help-rejecting complainer	Asks for help then rejects it	Control, fear of change	Move from advice to the function: "What would it cost you to accept help?"
Group mascot	Defuses everything with humour	Avoids grief	"Your jokes are welcome. I also want to know what is underneath this one."
Scapegoat	Becomes the group's problem	Carries disowned material	Protect; interpret the group's function in selecting them
Subgroup	Two or three form a bloc	Safety in numbers	Bring it into the open; do not let it be secret

The principle. Every difficult behaviour is a solution the member has found for a problem. Your task is to understand the problem, not to eliminate the behaviour by force.

Week 10 Conflict, scapegoating and subgrouping

Escalation in groups is faster than in dyads because of contagion. Interrupt earlier than you would in a family session.

The scapegoat sequence (learn to spot it): a member is different the group's anxiety attaches to them the group pressures them the leader is invited to condemn them if the leader agrees, the group learns that difference is dangerous.

Leader intervention: name the process, protect the member, and give the group the work: "I notice we have all become very interested in what Femi is doing wrong. What is it about him that makes us comfortable focusing there?"

Subgroups. Two members who meet outside, or a bloc that votes together. Name it without accusation: "I am aware that some of you talk outside. That is allowed, and it affects what happens here. What shall we do about it?"

Week 11 Co-facilitation

Why co-facilitate. Two sets of eyes, two relational styles, a model of a working relationship, and protection when one of you is triggered.

Division of labour decide before every session.

- Who opens and closes?
- Who tracks process, who tracks content?
- Who handles risk or a member leaving the room?
- How will you signal a disagreement in the moment?

Managing disagreement in front of the group. Never contradict your co-leader without acknowledging it. Options: (a) defer and discuss after; (b) name the difference openly and model resolution: "I see it differently, and I want the group to hear both." Naming it well is a gift; naming it badly is a rupture.

The classic co-therapy failure: the couple dynamic. Two co-leaders who replicate a parent child or pursue withdraw pattern in front of the group. The group will notice and may act it out. Take it to supervision immediately.

> **PRACTICAL**
> Co-therapy pairs plan a session in ten minutes using the division-of-labour checklist, then facilitate with the observed group. The observer records every moment the pair were out of sync.

Week 12 Observed co-facilitation

Thirty minutes of an experiential group, co-facilitated, assessed against: safety and structure; use of process; inclusion of silent members; containment of difficult behaviour; co-leader coordination; and closing.

Week 13 Couples and parents groups

Couples group. Four to six couples. Strengths: normalisation ("other couples have this too"), vicarious learning, and reduced shame. Risks: public conflict, competition, and one partner feeling ganged up on.

Structure: 15 minutes check-in (one sentence per person), 20 minutes teaching, 30 minutes exercise (couples work together while leaders circulate), 15 minutes group debrief. Never ask a couple to do a deep vulnerability exercise in front of others before they have done it alone with you.

Parents group. Strengths: reducing isolation, shared problem-solving. Risks: one parent becoming the group's expert, and advice-giving replacing reflection.

Structure: check-in, one skill, one application, one commitment. Keep advice horizontal: "Who has tried something that worked?"

Week 14 Ethics and boundaries in groups

Issue	Standard
Confidentiality	Explain it is a norm, not a guarantee; revisit when a member leaves
Screening	Never skip it; document it
Dual relationships	Members who know each other; leaders who know members socially
Touch	Discourage between members; use only with explicit norms
Out-of-group contact	Agree a policy; do not become a member's confidant
Recordings	Separate consent from each member; a member may withdraw at any time
Risk disclosure	Leaders follow their duty of care; tell the group if confidentiality must be broken

The most common ethical complaint in groups is not dramatic: it is a member learning that something they said was repeated outside. Prevention: explicit norms, a written agreement, and a leader who models discretion.

Week 15 Evaluation and endings

Minimum evaluation package. Attendance and drop-out (with reasons), a pre/post measure of the target problem, a session-by-session satisfaction rating, and a follow-up at 8 12 weeks.

Writing up. Report the design, the actual membership, the adaptations you made, the outcome and the limitations. A well-reported small group is a contribution; an unreported one is a private experience.

Glossary

Cohesion the group's sense of belonging and mutual commitment; a precondition for productive work. **Group-as-a-whole** the group considered as a single system with its own dynamics. **Here-and-now** the immediate interpersonal process in the room, as distinct from outside content. **Mutual aid** the reciprocal helping that occurs between members. **Process** the relational layer of communication, as distinct from content. **Scapegoating** the group's displacement of anxiety onto one member. **Social microcosm** the group as a small society in which members' characteristic patterns appear. **Subgrouping** the formation of alliances within the group. **Therapeutic factor** a mechanism of change identified by Yalom (e.g. universality, cohesiveness).

Self-assessment

Multiple choice

1. Mutual aid refers to: (a) the leader helping members (b) members helping each other (c) financial support (d) group cohesion

2. Scapegoating in a group usually functions to: (a) help the scapegoat (b) absorb the group's anxiety (c) improve cohesion (d) test the leader

3. Pre-group preparation: (a) is optional (b) reduces drop-out and improves outcome (c) increases anxiety (d) is only for research

4. The leader's main task in the performing stage is to: (a) direct more (b) step back and let mutual aid work (c) introduce new rules (d) end the group

5. "Here-and-now" work means: (a) discussing childhood (b) working with immediate in-room process (c) using mindfulness (d) homework review

6. Confidentiality in groups is: (a) guaranteed (b) a norm that cannot be guaranteed (c) irrelevant (d) the leader's legal duty only

7. A closed group: (a) admits new members (b) has fixed membership (c) has no rules (d) is always shorter

8. Catharsis without integration typically produces: (a) lasting change (b) temporary relief (c) increased cohesion (d) no effect at all

9. The best composition decision when two members are in an active conflict is to: (a) include both to work it out (b) avoid placing them in the same group (c) seat them together (d) ask them to sign a contract

10. Parallel process in co-facilitation means: (a) the leaders take turns (b) the leaders' relationship replicates a family pattern (c) members copy the leaders (d) two groups run at once

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. List five design decisions and the rationale for each.
2. Name five therapeutic factors and one leader behaviour that strengthens each.
3. Give the four steps of process commentary with an example.
4. How would you intervene with a monopoliser without shaming them?
5. What are the four elements of a group evaluation plan?

Essay (choose one, 1,800 words)

1. Design a couples group for eight distressed couples, justifying every design decision.
2. "The group is the treatment, not the leader." Discuss with reference to mutual aid and process.
3. Analyse a difficult moment in your experiential group and set out what you would do differently as leader.

Further reading

- Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books.
- Bion, W. R. (1961). Experiences in Groups. Tavistock.
- Burlingame, G. M., Strauss, B., & Joyce, A. S. (2013). Change mechanisms and effectiveness of small group treatments. In Bergin and Garfield's Handbook.
- Asen, E., & Scholz, M. (2010). Multi-Family Therapy: Concepts and Techniques. Routledge.

Next: MFT 710 Couples Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 10 OF 28

MFT 710

Couples Therapy

16 teaching weeks 1 practical 4 worked cases 6 figures

SEMESTER TWO

1.71 units Prerequisite: MFT 702, 706

> Couple work is the largest single referral stream in most family therapy practices and the most demanding therapeutically: two people, both of whom want you to agree with them.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 710 Couples Therapy

MSc Marriage and Family Therapy Semester Two 1.71 Units 16 teaching weeks Prerequisite: MFT 702, 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 710 Couples Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	**Couples Therapy**	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The sixteen-week plan . . . 6

Week 1 What couples want, and what therapy offers . . . 6

Week 2 Assessment I . . . 7

Week 3 Individual sessions, IPV screening and secrets . . . 7

Week 4 The negative cycle . . . 8

Week 5 Attachment applied to couples . . . 9

Week 6 EFT I: tracking and de-escalation . . . 10

Week 7 Observed couple assessment . . . 11

Week 8 EFT II: restructuring . . . 11

Week 9 Behavioural couple therapy . . . 12

Week 10 The Gottman research programme . . . 13

Week 11 Infidelity and betrayal trauma . . . 14

Week 12 Sex and intimacy . . . 15

Week 13 Violence, control and contraindications . . . 16

Week 14 Separation, divorce and stepfamilies . . . 16

Week 15 Culture, faith and family of origin . . . 16

Week 16 Consolidation and ending . . . 17

Glossary . . . 17

Self-assessment . . . 17

- Multiple choice . . . 17
- Answer key . . . 18
- Short answer . . . 18
- Essay (choose one, 2,000 words) . . . 18

Further reading . . . 18

Course leader's summary. Couple work is the largest single referral stream in most family therapy practices and the most demanding therapeutically: two people, both of whom want you to agree with them. This course integrates attachment-based (EFT), behavioural (TBCT/IBCT) and systemic approaches, and takes infidelity, violence, separation and cultural context seriously.

How to use this book

Couple work is where the therapist's own relationship history is most easily activated. The reflective tasks in this book are not optional extras; they are the difference between a clinician who can hold two perspectives at once and one who quietly takes sides. Do them.

1. Course description

The course covers: assessment of the couple (including individual sessions and violence screening), the major evidence-based models (EFT and behavioural couple therapy), the Gottman research programme, communication training, infidelity and betrayal trauma, sexual problems, separation and divorce work, and cultural and religious context.

Three frameworks are integrated: **attachment** (why the bond matters), **behaviour** (what maintains the pattern), and **system** (how the pattern organises itself).

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a couple assessment safely, including IPV screening and individual sessions	Observed assessment
LO2	Formulate the couple's negative cycle in attachment terms	Case assignment
LO3	Deliver core EFT and behavioural interventions under supervision	Observed sessions
LO4	Work with infidelity, including disclosure, meaning and rebuilding	Case study
LO5	Address sexual concerns within the couple frame	Exam
LO6	Manage separation, divorce and stepfamily transitions ethically	Case study
LO7	Adapt couple work to cultural, religious and extended-family context	Viva

3. Assessment

Component	Weight	Due
Couple assessment observed (40 min)	25%	Week 7
Formulation and treatment plan (2,000 words)	25%	Week 10
Recorded or live session with analysis (30 min)	30%	Week 15

Component	Weight	Due
Examination	20%	Week 16

4. The sixteen-week plan

Week	Topic	Practical
1	What couples want and what therapy offers; the evidence	Map a couple's goals
2	Assessment I: structure, history, and the couple narrative	Intake role-play
3	Assessment II: individual sessions, IPV screening, secrets	Screening rehearsal
4	The negative cycle: pursue withdraw and its variants	Cycle mapping
5	Attachment theory applied to couples	Attachment interview
6	EFT I: tracking and de-escalation	Enactment drill
7	**Observed couple assessment**	Assessed
8	EFT II: restructuring and softening events	Softening rehearsal
9	Behavioural couple therapy: exchange and communication	Speaker listener
10	**Formulation and plan due**; the Gottman programme	Four horsemen practice
11	Infidelity and betrayal trauma	Disclosure planning
12	Sex and intimacy in couple therapy	Sensate focus design
13	Violence, control and when couple therapy is contraindicated	Safety planning
14	Separation, divorce, co-parenting and stepfamilies	Co-parenting mediation
15	Culture, faith and family-of-origin in couple work	Cultural adaptation
16	Consolidation, relapse prevention and ending	**Session analysis due**

Week 1 What couples want, and what therapy offers

Couples arrive with a presenting complaint (money, sex, in-laws, phone) and an underlying bond question ("are you there for me?"). The complaint is the doorway; the bond is the room.

The evidence, briefly. Couple therapy is effective for relationship distress with moderate-to-large effects. Behavioural couple therapy, cognitive-behavioural couple therapy, and emotionally focused therapy have the strongest evidence base. Emotionally focused therapy has the most consistent evidence for durability, including in high-distress couples.

What predicts a good outcome. Early change, motivation of both partners, shared definition of the problem, low coercive control, and consistently the quality of the shared therapeutic alliance.

What predicts failure. Coercive control, undisclosed affairs that surface mid-treatment, untreated addiction, severe individual psychopathology, and one partner who has already decided to leave but has not said so.

> **REFLECT**
> What did your own family teach you about conflict, repair, and what a marriage is for? Write 400 words. Keep it; you will return to it in Week 13.

Week 2 Assessment I

Structure of a couple assessment (2 3 sessions).

1. **Joint session:** both partners' accounts, the history of the relationship, the current pattern, and the goals.
2. **Individual sessions:** one with each partner risk screen, secrets policy, individual history, ambivalence, and "is there anything you would not say in front of your partner?"
3. **Feedback session:** formulation, plan, and the agreement.

What to assess.

Domain	Questions
Commitment	"Is there any part of you that has already decided to leave?"
History	How did you meet? What drew you? What was good in the early years?
Current pattern	"Walk me through last Tuesday's argument, minute by minute."
Attempted solutions	What have you tried? What happened?
Family of origin	Genogram of both families; how each family handled conflict, affection, money
Context	Work, money, migration, in-laws, children, faith, health
Strengths	What still works? What do you still enjoy?
Risk	Violence, coercive control, suicide, substance use, child safety

Ask about the good. Couples in distress systematically under-report positives; a careful strengths inventory is both assessment and intervention.

Week 3 Individual sessions, IPV screening and secrets

Always see each partner alone at least once. This is where safety is disclosed.

Screening questions (ask directly, in private).

- "Have you ever felt afraid of your partner?"
- "Has your partner ever hit, pushed, slapped, or thrown something at you?"

- "Has your partner ever forced or pressured you sexually?"
- "Does your partner control your money, your phone, or who you see?"
- "If you went home tonight and told him you had said all this, what would happen?"

If coercive control is present: stop conjoint work; safety-plan individually; refer to specialist services; document. Conjoint work can increase danger and can silence the victim in the room.

Secrets policy. State it in the first joint session: "If one of you tells me something individually that affects our work together, I will not keep it as a secret. I will help you decide how and when to tell your partner, and if you cannot, I will ask us all to talk about it."

Ambivalence. One partner may already be planning to leave. Ask: "Where are you on a scale from 'I am fully committed to working on this' to 'I have already decided'?" The answer changes the treatment.

Week 4 The negative cycle

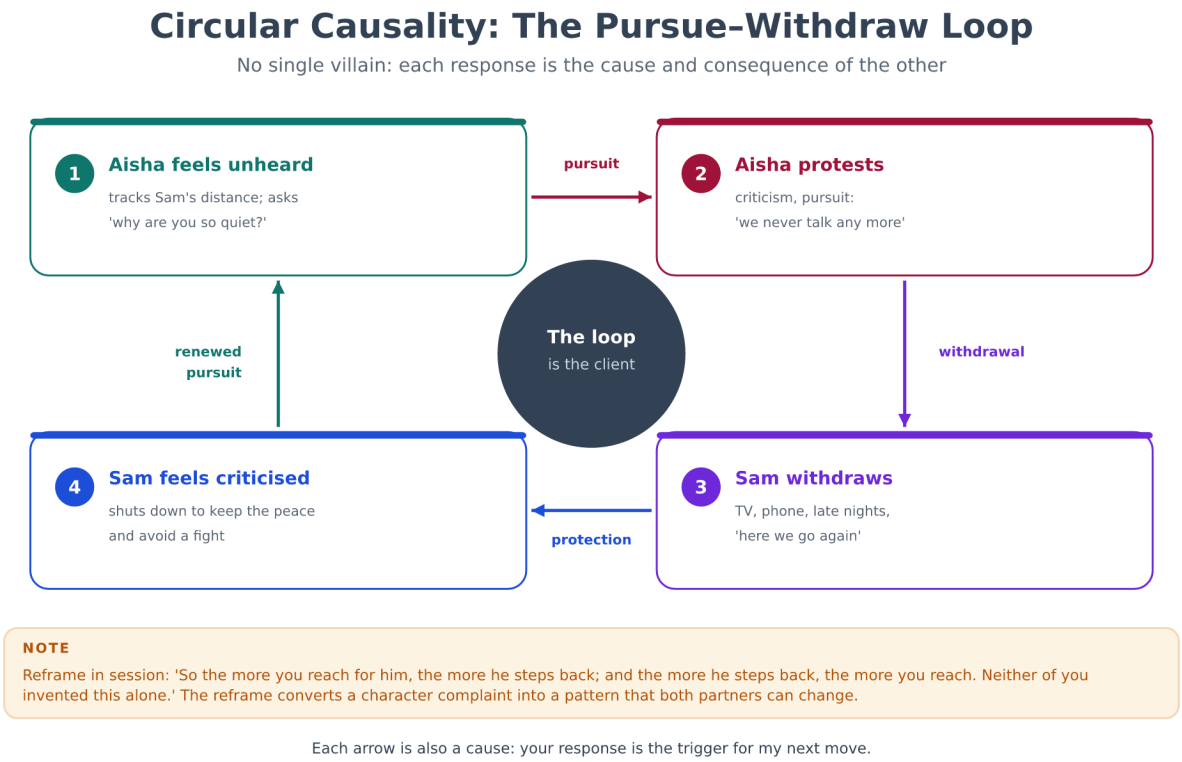


Figure 4.1 Circular causality: the pursue withdraw loop

The three common cycles.

1. **Pursue withdraw.** One escalates to get a response; the other shuts down to keep the peace. The pursuing increases, the withdrawal deepens.

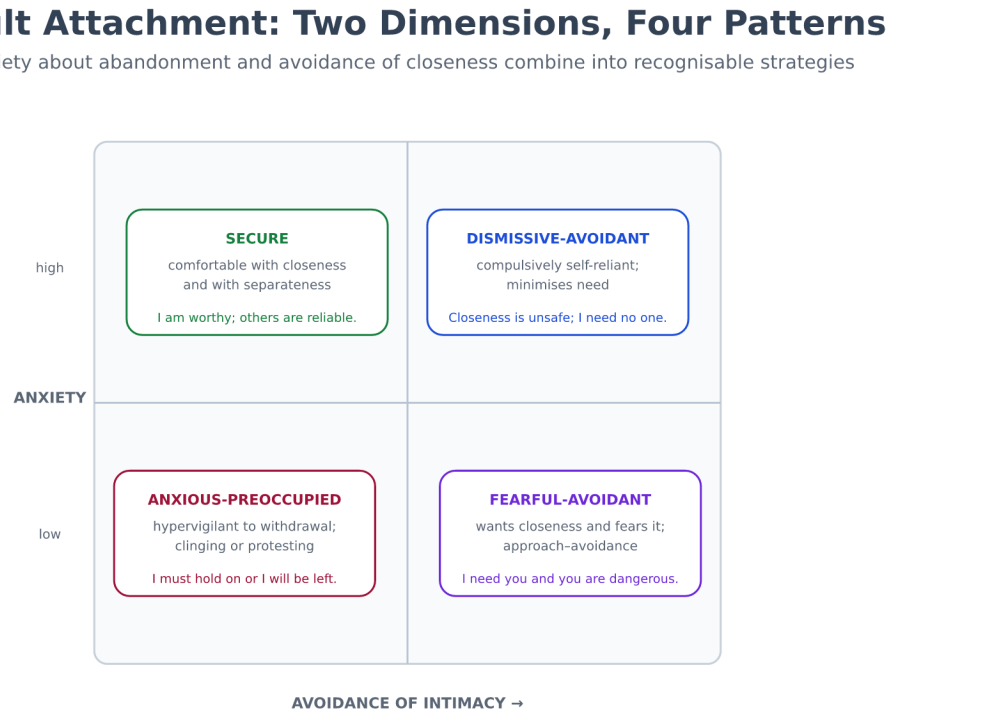
2. **Attack defend.** Both escalate; complaint is met with counter-complaint; the volume rises and the content becomes irrelevant.
3. **Mutual withdrawal.** Both avoid; the relationship becomes parallel and polite; loneliness without conflict often the hardest to treat because there is no presenting crisis.

Mapping the cycle in session. Ask each partner for the last three arguments and write the sequence on paper as a loop. Then read it back and ask: "Is this recognisable?" The moment of recognition often accompanied by a laugh or a sigh is the therapeutic turn.

The reframe. "The problem is not that you are angry, or that you are cold. The problem is the loop between you, and the loop has a life of its own."

> **PRACTICAL**
> With a role-played couple, map the cycle on a whiteboard in 15 minutes and get both partners to agree with the drawing. The agreement, not the drawing, is the skill.

Week 5 Attachment applied to couples



> **NOTE**
> Attachment is a pattern, not a personality type. The same person can be secure with a sibling and anxious with a partner, and patterns shift with context, therapy and a safe relationship. Assess the pattern, then ask what it protects.

Figure 5.1 Adult attachment: two dimensions and four patterns

Core idea. Adult romantic love is an attachment process. Partners monitor accessibility and responsiveness; when the bond feels threatened, protest behaviours appear (criticism, pursuit, withdrawal, numbing), and those protest behaviours are usually the opposite of the need they express.

Two dimensions. Anxiety (about abandonment) and avoidance (of intimacy). Their combination produces four patterns, each with a characteristic strategy and core belief (see figure).

Clinical use.

- Anxious partner: the protest is a request for reassurance. Help them ask for it directly rather than through criticism.
- Avoidant partner: withdrawal is a protection against overwhelm and shame. Help them stay present for longer and return rather than disappear.
- Both: the cycle is an attachment problem, not a character problem.

The attachment question to hold in your head. "What is this person asking for, and what are they doing instead?"

Week 6 EFT I: tracking and de-escalation

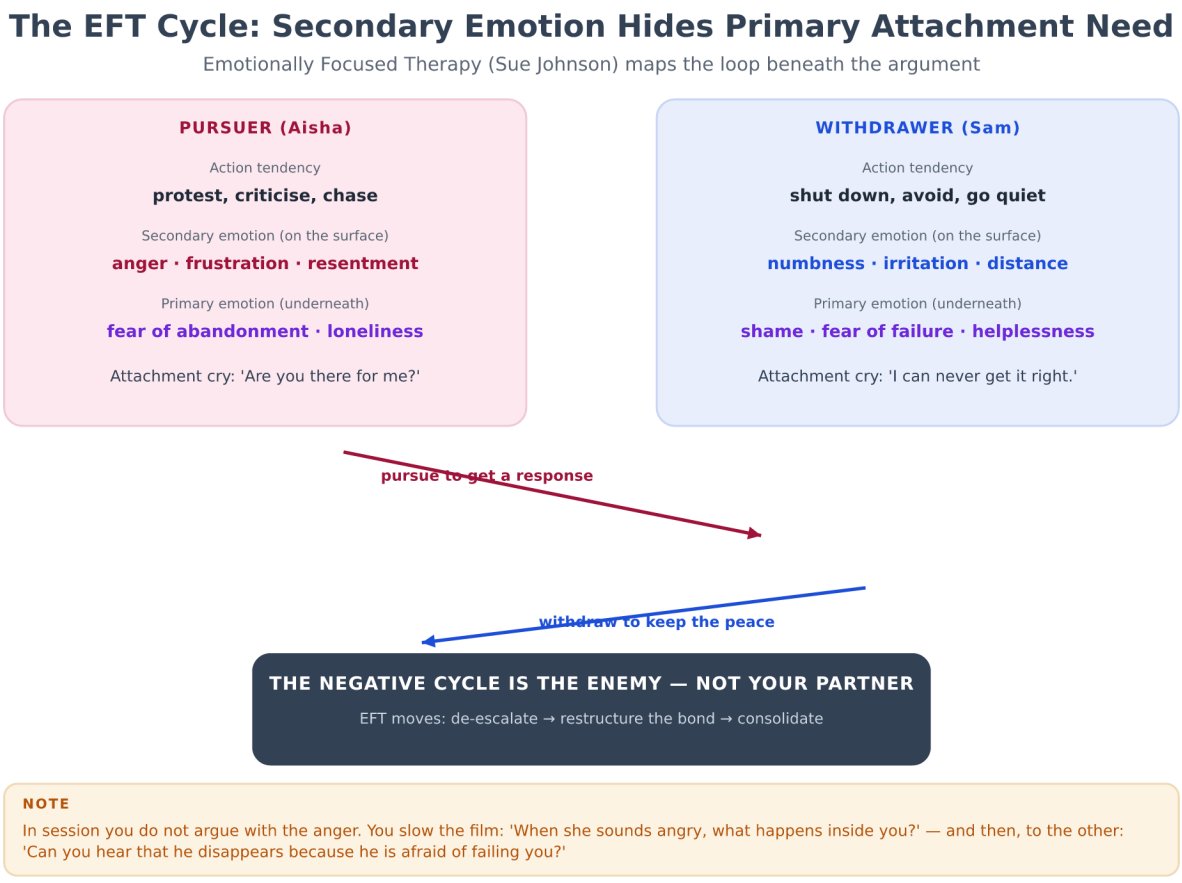


Figure 6.1 The EFT cycle: secondary emotion and primary attachment need

Stage 1 (de-escalation, sessions 1 8). The goal is that both partners can describe the cycle and see it as the enemy.

Four moves.

1. **Track the cycle** in specific incidents. Slow the film; ask for bodily experience.
2. **Access primary emotion.** "Under the anger is there something like fear?" Name it tentatively; let them correct you.
3. **Reframe the problem as the cycle.** "Neither of you is the problem. This dance is."
4. **Name the attachment cry.** "So when you say 'we never talk', what you mean is 'I miss you and I am afraid you have gone'."

A worked enactment (Week 6 lab).

> **THERAPIST (TO AISHA):**
> When you get loud, what happens inside your body? **Aisha:** My chest. Like I cannot breathe until he answers. **Therapist:** And if you did not get loud what would he see? **Aisha:** (pause) That I am frightened. **Therapist:** Can you tell him that? Look at him. "I get loud because " **Aisha:** I get loud because I think you have already left. **Therapist (to Sam):** Sam, stay with that. What is happening for you hearing it?

Hazard. Primary emotion must be reached, not demanded. If you push a client to feel something in front of you before safety, you will get a performance or a shutdown.

Week 7 Observed couple assessment

Forty minutes: contract, both accounts, cycle map, risk screen (in the individual sessions if scheduling permits), strengths, and feedback. Assessed on safety, structure, balance (did each partner get equal airtime?), and formulation quality.

Week 8 EFT II: restructuring

Stage 2 (sessions 9 20). Withdrawal re-engagement; pursuer softening; enactment of new responses.

Softening event the classic sequence.

1. The pursuer accesses the primary attachment fear and expresses it non-accusatorially.
2. The withdrawer hears it, stays present, and responds with care.
3. The pursuer risks reaching again from a position of vulnerability rather than protest.
4. Both experience a new pattern in the room.

Therapist tasks. Keep the withdrawer engaged ("stay with me, this is the hard part"); block the pursuer's escalation without silencing the need; and slow everything down.

Stage 3 (consolidation). Integrate the new pattern, problem-solve practical issues, and rehearse recovery from relapse: "When the cycle comes back and it will what will you do in the first hour?"

Week 9 Behavioural couple therapy

The behavioural view. Distress is maintained by (a) a low rate of positive exchange, (b) poor communication under conflict, and (c) attributions that make neutral behaviour hostile.

Core interventions.

1. **Behavioural exchange.** Each partner lists 15 20 small caring behaviours; select three each; do them daily without being asked. Not transactional bookkeeping an increase in the density of positive behaviour.
2. **Communication training.** Speaker listener with a floor card, then problemsolving.
3. **Attribution retraining.** "When he is late, what do you assume? What else could be true?"
4. **Acceptance work (IBCT).** Some differences will not change; the work is to soften the emotional reactivity to them through empathic joining and tolerance building.

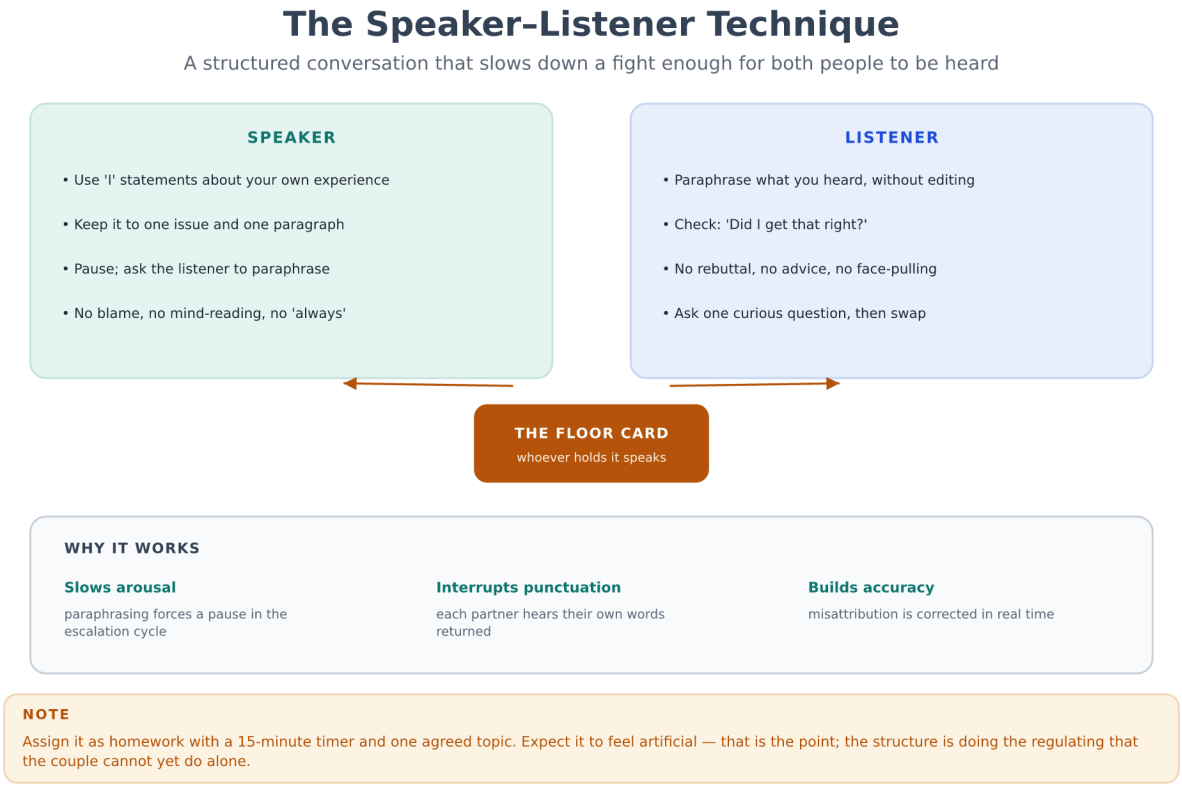
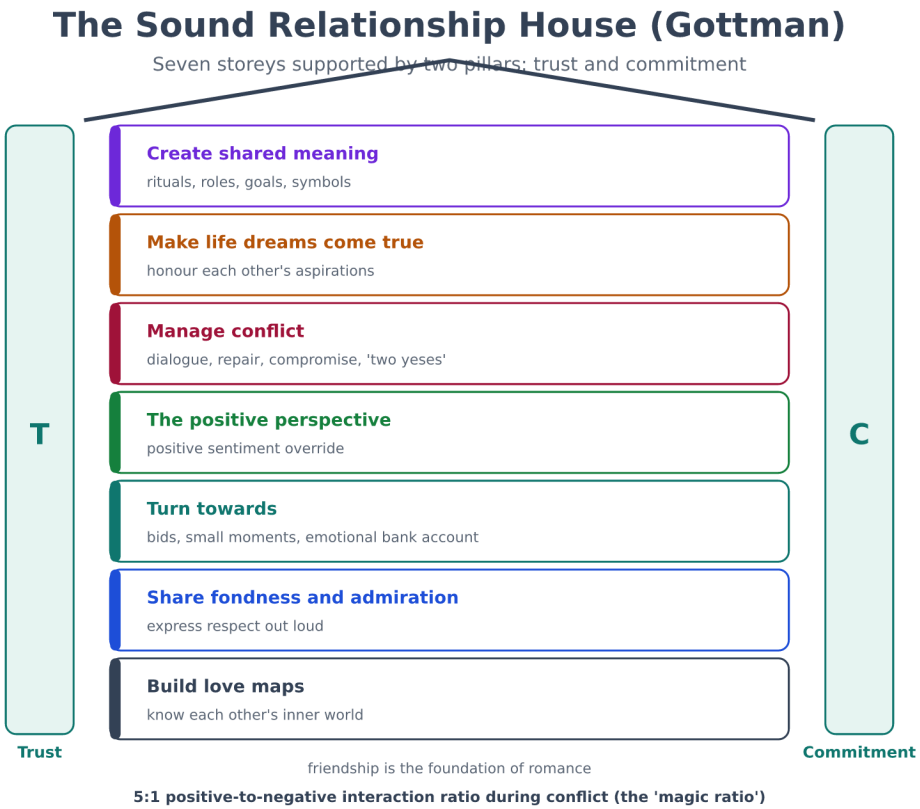


Figure 9.1 The speaker listener technique

Sequencing. Positives first (exchange), then communication, then problem-solving. Couples cannot solve problems they cannot discuss.

Week 10 The Gottman research programme



NOTE

Adaptation caution: Gottman's norms were derived largely from U.S. samples. Use the structure of the house as a clinical map; do not treat the 5:1 ratio as a universal law.

Figure 10.1 The Sound Relationship House

Findings worth using.

- **The four horsemen** criticism, contempt, defensiveness, stonewalling predict dissolution (Figure 14). Contempt is the strongest predictor.
- **The 5:1 ratio** of positive to negative interaction during conflict.
- **Bids for connection** small moments of "turning towards" build the emotional bank account.
- **Love maps** detailed knowledge of the partner's inner world.
- **Repair attempts** the capacity to de-escalate mid-conflict is more predictive than the absence of conflict.
- **Physiological flooding** above roughly 100 bpm, people cannot listen; a break is a skill, not an escape.

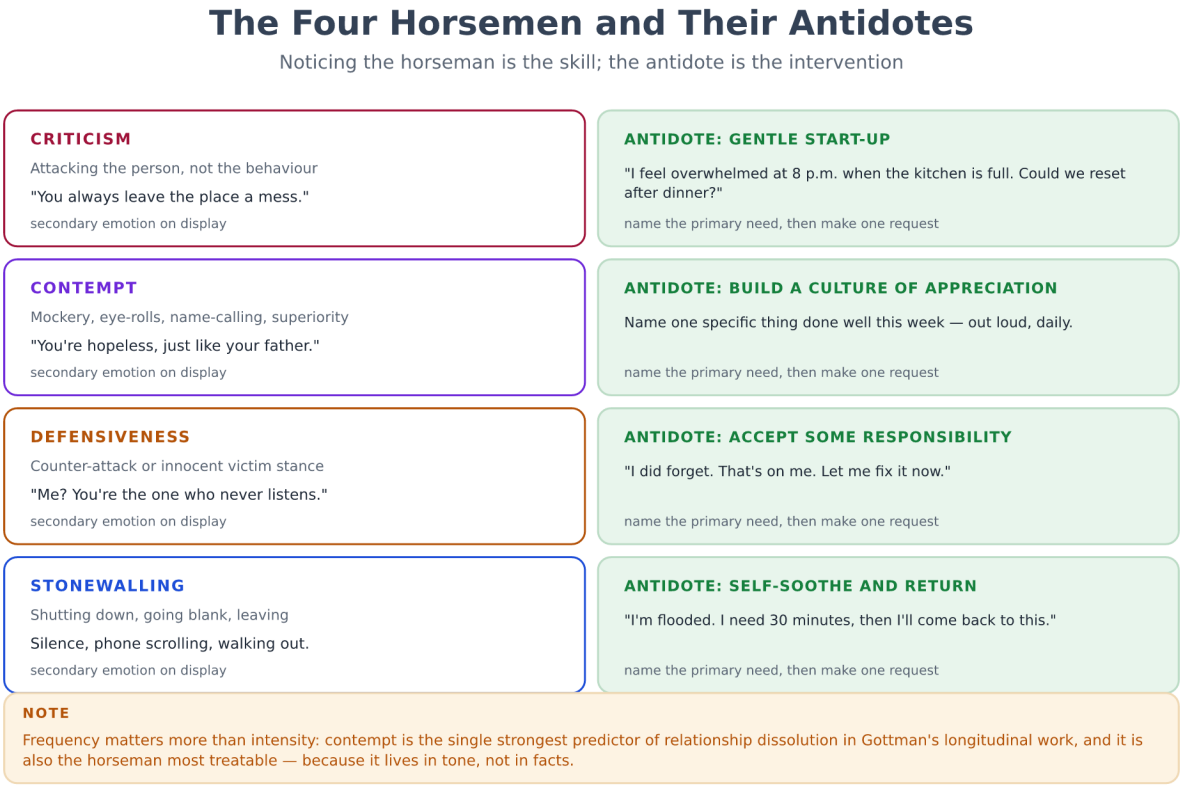


Figure 10.2 The four horsemen and their antidotes

Adaptation caution. The norms come largely from US samples; treat the structure as a clinical map, not as a universal law. A couple who argue loudly and repair well is not in trouble because of the volume.

Week 11 Infidelity and betrayal trauma

Two clinical events, often confused.

1. **The disclosure** an acute crisis requiring containment, safety and structure.
2. **The betrayal trauma** an injury to the betrayed partner's capacity to trust and to make sense of reality, with symptoms resembling PTSD (intrusion, hypervigilance, dissociation).

Phase 1: stabilise (weeks 1 4).

- Stop the affair (or the contact) this is usually a precondition, not a negotiation.
- Establish safety: no violence, no coercion, agreed boundaries on phone and whereabouts.
- Contain the disclosure: the betrayed partner has a right to the facts, not to every detail; agree what will be asked and answered.
- Address STI testing and any financial/legal exposure.
- Individual sessions to assess each partner's capacity to continue.

Phase 2: meaning (weeks 4 12).

- What did the affair do for the unfaithful partner? (Not "why did you do it?" but "what was it in service of?")
- What was missing that could have been spoken? (Careful: explanation is not justification.)
- What does the betrayed partner need in order to consider rebuilding transparency, remorse, time, consistency?

Phase 3: rebuilding or separating. Not every couple should stay together. Your task is to help them decide with full information, not to preserve the marriage at all costs.

> **CAUTION**
> Do not accept a couple for affair work if the unfaithful partner will not end the affair and will not disclose anything; you will be running a therapy that cannot progress and colluding in a lie.

Week 12 Sex and intimacy

Start with permission, then information, then specific suggestions (Annon's PLISSIT model).

Assessment. Desire, arousal, orgasm, pain; frequency and satisfaction for each partner; the last time it was good; what changed; medical factors (medication, illness, menopause, postpartum); the couple's beliefs; and the meaning of sex in the relationship.

Common presentations and the couple-level formulation.

Presentation	Individual factor	Relational factor
Low desire in one partner	Fatigue, medication, depression, pain	Pursuit and pressure; unresolved resentment
Erectile difficulty	Vascular, hormonal, anxiety	Performance focus; fear of judgement
Painful intercourse	Medical cause must be excluded	Fear, poor communication, coercion
Mismatched desire	Cycles differ normally	Negotiation failure; rejection sensitivity

Intervention basics. Psychoeducation (desire discrepancy is the most common sexual complaint, not a sign of incompatibility); sensate focus (graded, non-goal-oriented touch); scheduling; reducing performance pressure; addressing the relational context; and medical referral when indicated.

Cultural and religious framing. Sexual expectations are among the most culturally loaded topics you will discuss. Ask what each partner was taught, what they believe is permitted, and whether they can say no to each other. Consent within marriage is consent.

Week 13 Violence, control and contraindications

The distinction that matters (from MFT 704):

Situational couple violence	Coercive control
Mutual, escalating under stress	One-directional domination
No fear outside conflict	Chronic fear, surveillance, isolation
Couple therapy possible after assessment	Couple therapy contraindicated
Safety plan for both	Specialist advocacy for the victim

If a couple presents with violence. See each partner individually first. Ask about fear, injury, sexual coercion, threats, and escalation. Do not use conjoint sessions to "work through" violence; do not ask the victim to say in front of the perpetrator what he did, unless she chooses and it is safe.

REFLECT Return to your Week 1 reflection. Where in this material are you most likely to lose balance with the victim, with the perpetrator, with the unfaithful partner, with the one who wants to leave?

Week 14 Separation, divorce and stepfamilies

Discernment counselling. Some couples are not deciding whether to repair the relationship but whether to end it. A structured, time-limited process (about 4 6 sessions) helps a couple reach a clear decision rather than drifting. Both outcomes a renewed commitment or a respectful ending are successes.

When the decision is to separate.

- Help the couple tell the children together, with a simple, blame-free message.
- Refer for legal advice on custody, maintenance, property and any customary or religious processes.
- Address co-parenting: the relationship continues as a parenting partnership.
- Manage the grief: divorce is a loss, even when it is a relief.

Stepfamilies. The central error is treating the stepfamily as a nuclear family that arrived late. It is a different structure with different tasks: the couple bond must be built before stepparent authority is imposed; children need permission to love both households; and the "insider/outsider" split (biological parent and child versus stepparent) must be named.

Week 15 Culture, faith and family of origin

Questions to ask every couple.

- What did each of your families teach you about marriage, money, sex, and in-laws?
- Who has authority in your household, and who decided that?

- What does your faith require of each of you?
- Which of your families is closer, and what does that closeness cost the other?
- If your parents disagreed with a decision you made, what would happen?

Common cultural configurations. Bride price and its obligations; polygyny; co-residence with parents; gender-role expectations; religious courts and counselling; migration and split households; inter-ethnic and inter-faith marriages.

The rule. Do not treat the cultural pattern as the problem, and do not treat it as beyond discussion. Ask the couple what they want within it.

Week 16 Consolidation and ending

Write the couple a one-page **relapse prevention plan**: their cycle in their own words, three early warning signs, two repair moves that work for them, and the agreement about when to return. Review the gains explicitly, name the risk of recurrence without frightening them, and end on time.

Glossary

Attachment injury a betrayal or abandonment that damages trust and requires explicit repair. **Betrayal trauma** trauma-like symptoms following the discovery of a partner's deception. **Bids for connection** small attempts to gain a partner's attention or affection. **Coercive control** a pattern of domination and isolation, distinct from situational conflict. **Negative cycle** the recurrent pattern of interaction that organises couple distress. **Pursue withdraw** a cycle in which one partner escalates while the other distances. **Softening** in EFT, the pursuer's shift from protest to vulnerable expression. **Sentiment override** the tendency for the relationship's overall tone to colour the interpretation of specific events. **Sensate focus** graded, non-goal-oriented touch exercises used in sex therapy.

Self-assessment

Multiple choice

1. The first step in assessing a couple is: (a) communication training (b) assessment including individual sessions and IPV screening (c) homework (d) diagnosis

2. In EFT, criticism is usually understood as: (a) abuse (b) protest at perceived abandonment (c) personality (d) a communication deficit

3. Couple therapy is contraindicated when: (a) the couple argues (b) there is coercive control (c) there is low desire (d) one partner is depressed

4. The strongest predictor of relationship dissolution in Gottman's research is: (a) conflict frequency (b) contempt (c) low income (d) different interests

5. A 5:1 ratio refers to: (a) sessions per month (b) positive to negative interactions during conflict (c) partners to therapist (d) years of marriage

6. In affair work, the precondition for progress is usually: (a) forgiveness (b) ending the affair and establishing transparency (c) a legal separation (d) a new sexual contract

7. "Softening" in EFT refers to: (a) lowering the volume (b) the pursuer expressing primary attachment emotion (c) the therapist's tone (d) reducing homework

8. Physiological flooding is significant because: (a) it indicates anger (b) above a threshold, listening becomes physiologically difficult (c) it predicts divorce (d) it requires medication

9. A key task in stepfamilies is to: (a) impose stepparent authority quickly (b) build the couple bond before stepparent authority (c) treat all children equally in discipline (d) avoid the ex-partner

10. Discernment counselling helps couples to: (a) stay together (b) reach a clear decision about the relationship (c) divorce quickly (d) avoid decisions

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Give the three-session structure of a couple assessment and the purpose of each session.
2. Write your confidentiality policy for secrets in couple therapy, in six sentences.
3. Describe the pursue withdraw cycle in attachment terms.
4. Name three contraindications to conjoint couple therapy.
5. What are the phases of infidelity work, and what happens in each?

Essay (choose one, 2,000 words)

1. Compare EFT and behavioural couple therapy for a highly distressed couple with a pursue withdraw cycle.
2. "Infidelity is a symptom and an injury." Discuss the treatment implications.
3. Critically evaluate the cultural assumptions embedded in the concept of "differentiation" in couple therapy.

Further reading

- Johnson, S. M. (2019). Attachment Theory in Practice. Guilford.
- Gottman, J. M., & Gottman, J. S. (2015). 10 Principles for Making Marriage Work. Harmony.

- Epstein, N. B., & Baucom, D. H. (2002). Enhanced Cognitive-Behavioral Therapy for Couples. APA.
- Snyder, D. K., Baucom, D. H., & Gordon, K. C. (2007). Getting Past the Affair. Guilford.
- Doherty, W. J. (2013). Take Back Your Marriage. Guilford.
- Lebow, J., & Snyder, D. K. (2022). Clinical Handbook of Couple Therapy (6th ed.). Guilford.

Next: MFT 711 Child and Adolescent Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 11 OF 28

MFT 711

Child and Adolescent Therapy

15 teaching weeks 3 practicals 1 figure

SEMESTER TWO

1.71 units Prerequisite: MFT 703, 706

> Children rarely refer themselves and rarely benefit from an adult conversation.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0

MFT 711 Child and Adolescent Therapy

MSc Marriage and Family Therapy Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 703, 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 711 Child and Adolescent Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	**Child and Adolescent Therapy**	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 Children in context . . . 6

Week 2 Developmental assessment and red flags . . . 7

Week 3 Engaging children . . . 8

Week 4 Engaging adolescents . . . 9

Week 5 Consent, confidentiality and divided loyalty . . . 9

Week 6 Anxiety and school refusal . . . 10

Week 7 Behaviour problems and parenting . . . 10

Week 8 Observed child assessment . . . 11

Week 9 Trauma and maltreatment . . . 11

Week 10 Self-harm and suicidality in adolescents . . . 11

Week 11 Divorce, separation and blended families . . . 12

Week 12 Eating difficulties . . . 12

Week 13 Schools and paediatric services . . . 12

Week 14 Neurodivergence in family context . . . 13

Week 15 Endings with children . . . 13

Glossary . . . 14

Self-assessment . . . 14

- Multiple choice . . . 14
- Answer key . . . 14
- Short answer . . . 14
- Essay (choose one, 2,000 words) . . . 15

Further reading . . . 15

Course leader's summary. Children rarely refer themselves and rarely benefit from an adult conversation. This course teaches you to work with children as agents in a family system: how to speak to them, play with them, assess developmentally, and intervene through the adults who live with them.

How to use this book

Each week has a **developmental frame** (what is normal at this age), a **clinical frame** (what is different when it goes wrong), and a **practice frame** (what you actually do in the room). The play and activity protocols are written so you can use them next week, not after you qualify.

1. Course description

Child and adolescent work in family therapy is not a specialism bolted on to adult work; it is a distinct skill set. Children communicate through play, behaviour and the body; adolescents communicate through testing, silence and peers; and both live inside systems that can either support or undermine change.

The course covers: developmental assessment, engaging children and adolescents, family-based interventions, common presentations (anxiety, behaviour problems, school refusal, trauma, self-harm, eating difficulties), divorce and blended families, safeguarding, and the interface with schools and paediatrics.

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a developmental and systemic assessment of a child or adolescent	Observed session
LO2	Engage a child using play, drawing and conversation appropriate to age	Skills assessment
LO3	Formulate a child's presentation relationally and developmentally	Case study
LO4	Deliver a family-based intervention for at least two common presentations	Case study
LO5	Work with schools, paediatricians and child protection systems	Report
LO6	Recognise and respond to safeguarding concerns	Exam
LO7	Manage the ethical complexities of consent, confidentiality and divided loyalty	Viva

3. Assessment

Component	Weight	Due
Child assessment observed (30 min with child + 20 min feedback to carers)	30%	Week 8

Component	Weight	Due
Family-based intervention case study (2,500 words)	40%	Week 13
Examination	30%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Children in context: why family therapy is child therapy	Observation of a child at play
2	Developmental assessment and red flags	Milestone interview
3	Engaging children: language, play, drawing, behaviour	Drawing interview
4	Engaging adolescents: alliance, autonomy, honesty	Adolescent interview
5	Consent, confidentiality and divided loyalty	Ethics scenario work
6	Anxiety and school refusal	Family-based exposure plan
7	Behaviour problems and parenting interventions	Parent session rehearsal
8	**Child assessment observed**	Assessed
9	Trauma and maltreatment in children	Safety and stabilisation
10	Self-harm and suicidality in adolescents	Risk assessment with a young person
11	Divorce, separation and blended families	Telling-the-children protocol
12	Eating difficulties and body image	Family-based approach outline
13	**Case study due**; working with schools and paediatric services	Report writing
14	Neurodivergence in family context	Environmental adaptation
15	Endings with children; integration	Termination play session

Week 1 Children in context

The paradox of child therapy. A child is referred because of behaviour. The behaviour occurs in a context. If you treat the child alone, you have removed them from the context that maintains the behaviour and asked them to change it on their own. Family-based work is therefore not an ideological preference; it is a practical inference.

Children as agents. Children are not passive recipients of family dynamics. They notice, interpret, protect, sacrifice and resist. A nine-year-old who refuses school may be keeping a depressed mother company; a thirteen-year-old who fights may be absorbing a father's rage to protect a younger sibling.

The evidence. Family-based treatments have strong evidence for: conduct problems (parent management training, functional family therapy, multisystemic therapy),

adolescent substance misuse, eating disorders (family-based treatment for anorexia nervosa is first-line), anxiety in children (parent-involved CBT), and psychosis relapse prevention (family intervention).

PRACTICAL

Observe a child (with consent) for 15 minutes in free play. Record only behaviour: what they did, in what order, what they returned to. No interpretation for the first ten minutes.

Week 2 Developmental assessment and red flags



Figure 2.1 Developmental milestones from 0 to 18

The four-category check (from MFT 703): normal variation, delay, regression, symptom.

Red flags requiring referral, not therapy alone.

Domain	Red flag
Language	Loss of words; no words at 24 months with no comprehension
Motor	Loss of motor skills; persistent asymmetry; regression
Social	Loss of eye contact and social interest; loss of pretend play

Domain	Red flag
Behaviour	Sudden change in personality; new aggression after a specific event
School	Marked decline in a previously competent child
Medical	Seizures, severe headaches, weight loss, unexplained bruising
Mental state	Hearing voices; fixed beliefs; severe withdrawal; self-harm
Safeguarding	Disclosure, injuries, sexualised behaviour, fear of a specific adult

Assessment with children. Use several modes: interview, drawing, play observation, parent report, teacher report, and standardised measures (SDQ, RCADS). Children's self-report and parents' report diverge systematically collect both, report both.

Week 3 Engaging children

Rules of engagement.

1. **Explain the frame to the child, not just the adult.** "We will meet four times. We will talk and sometimes draw. You can say 'pass' to any question."
2. **Get on their level.** Sit on the floor, use their name, match their pace.
3. **Use a medium.** Drawing, puppets, sand, figures, or a simple game. The medium is not a distraction; it is the language.
4. **Do not interrogate.** One question, then follow. "Tell me about this house" beats "who lives here?"
5. **Be honest.** Children detect adult pretence and are damaged by it. If you cannot answer, say so.

Three reliable techniques.

- **The three wishes.** "If you had three wishes, what would they be?" (One is usually for the child, one for the family, one impossible.)
- **Draw your family.** Then ask: "Who is this? What are they doing? Who is closest to whom?" Note who is omitted, who is drawn largest, who is at the edge.
- **The magic wand / miracle.** "If you woke up and the problem was gone, what would be different at breakfast?"

Avoid interpretation. "That must be your father" is a projection. Ask instead: "Tell me about this person."

> **PRACTICAL**
> 20-minute drawing interview with a peer playing an 8-year-old. Use the three techniques. Debrief: how many questions did you ask, and what did you learn without asking?

Week 4 Engaging adolescents

What adolescents need from a therapist. Honesty, respect for autonomy, a genuine alliance with them (not with their parents), and confidentiality that is real except where safety requires otherwise.

The first five minutes. Speak to the adolescent first. Ask their version before the parents'. Explain confidentiality in their language: "What you tell me stays here, unless I think you or someone else is in danger. If that happens, I will tell you before I tell anyone else."

Techniques that work.

- **Third-person questions.** "What do teenagers your age usually say about this?" Lowers the exposure.
- **Scaling.** "On 0 10, how much does this bother you? How much does it bother your mum?"
- **Future questioning.** "What do you want your life to look like in two years? What would have to change first?"
- **Silence tolerance.** Do not fill every pause.
- **Negotiated goals.** "What would make this worth your time?"

What fails. Lectures, moralising, siding with parents, pretending to be young, and asking "why".

Week 5 Consent, confidentiality and divided loyalty

Consent. For minors, a parent or guardian consents; the young person **assents**. Both matter. Record both. Where a competent adolescent seeks help without parental knowledge, follow your jurisdiction's law and your agency policy and do not improvise.

Confidentiality with children.

Situation	Approach
Child discloses something minor	Keep it; help them tell the parent if they want
Child discloses self-harm	Assess risk; agree a plan; inform carers with the child's knowledge
Child discloses abuse	Follow safeguarding procedure; tell the child what will happen next
Parent asks what the child said	Give themes, not transcripts; protect the alliance
Parent demands a report for court	Refer; a treating therapist should not provide forensic opinion

Divided loyalty. The most common clinical trap: the child tells you something that would hurt the parent, or the parent asks you to "fix" the child's attitude. Name the dynamic and refuse to be recruited: "I am not going to be the one who tells your mother. I will help you do it."

Week 6 Anxiety and school refusal

Anxiety in children. Developmentally normal fears (separation, darkness, monsters, social evaluation) become disorders when they are persistent, impairing and out of proportion.

The family-based approach.

1. **Psychoeducation.** Anxiety is a false alarm; avoidance reduces anxiety now and increases it later.
2. **Map the accommodation.** What does the family do to prevent the child's distress? (Answering questions for them, sleeping in their bed, allowing school avoidance, checking repeatedly.) Accommodation is usually maintained by love, and naming it must be done without blame.
3. **Graded exposure.** Together, build a ladder from easiest to hardest; start at the bottom; reward approach, not the absence of anxiety.
4. **Parental coaching.** Parents become coaches, not rescuers: "I know this is hard. I will be here at 3 o'clock."
5. **The school plan.** One named adult, a quiet space, a gradual return timetable, and no negotiation at the school gate.

School refusal a differential. Anxiety, separation difficulty, bullying, learning difficulty, family crisis or a bid to protect a parent. The treatment differs for each. Assess by asking what happens on Sunday night, and what the child's absence makes possible at home.

> **PRACTICAL**
> Build an exposure ladder with a role-played parent for a 10-year-old with separation anxiety. Ten rungs, specific, graded, with a reward and a review date.

Week 7 Behaviour problems and parenting

The evidence base. Parent management training (PMT) and its variants have the strongest evidence for child conduct problems. The mechanism is consistent: increase positive attention, give clear instructions, use consistent consequences, reduce harsh discipline, and rebuild the parent child relationship.

The core skills (teach, model, rehearse, review).

Skill	What it looks like
Special time	10 minutes daily of child-led play, no correction
Praise	Specific, immediate, frequent ("thank you for putting your shoes away")
Instructions	Get attention, get close, one instruction, allow 5 seconds, praise compliance
Planned ignoring	For minor attention-seeking; not for dangerous behaviour

Skill	What it looks like
Consequences	Consistent, proportionate, predictable, delivered calmly
Routine	Predictable structure reduces conflict

Why parenting programmes fail. Too much content per session; the parent is depressed or exhausted; the child has an untreated condition; another adult undermines the plan; or the family's material circumstances make consistency impossible. Address these before adding more technique.

Adaptation. In households where several adults care for the child (grandmother, aunt, house help), bring them all in and agree one plan. A plan implemented differently by four adults is not a plan.

Week 8 Observed child assessment

Thirty minutes with a child (role-played by a trained peer/actor), then twenty minutes of feedback to the carers. Assessed on: engagement, developmental appropriateness, information gathering without interrogation, risk screen, and the quality of the feedback.

Week 9 Trauma and maltreatment

Principles (developed in MFT 714). Safety first; stabilise before processing; the caregiver's own regulation is the child's best medicine; and never force a narrative.

What trauma looks like in children.

Age	Presentation
0 2	Feeding and sleep disruption, startled response, withdrawal
3 6	Regression, repetitive play, nightmares, clinging
7 11	Aggression, school problems, somatic complaints, guilt
12 18	Risk-taking, self-harm, dissociation, substance use, precocious sexuality

Family work after maltreatment. The non-offending caregiver's belief and protection is the single strongest predictor of recovery. Work with them first. Where the perpetrator remains in the home, safety planning precedes everything.

Safeguarding. Know your reporting route, document verbatim disclosures without leading questions, and never promise a child secrecy about abuse.

Week 10 Self-harm and suicidality in adolescents

Ask directly. "Sometimes when people feel this bad, they think about hurting themselves or ending their life. Has that happened for you?" Direct questions do not plant ideas.

Assess: frequency, method, function (relief, punishment, communication, control), intent, plan, means, protective factors, and the family's response.

Manage the family. Parents often respond with panic, punishment or secrecy. Help them respond with calm supervision and connection: remove means, increase monitoring without surveillance, and seek help immediately if risk rises.

Confidentiality. A young person's request for secrecy must yield to safety. Tell them what you will disclose and why, and involve them in how.

Week 11 Divorce, separation and blended families

Telling the children. Both parents together, a simple message with no blame, permission to love both parents, and reassurance that the adults' decision is not the child's fault. No details about the affair, the money or the courts.

What harms children. Exposure to conflict (more than separation itself), being used as a messenger, being asked to choose, hearing a parent denigrated, and instability.

What protects them. A cooperative co-parenting relationship, structure, a relationship with both parents, and adults who can manage their own distress.

Blended families. Build the couple bond first; stepparents take authority gradually and through the biological parent; children need permission to love both households; and the "insider/outsider" split must be named and worked with.

Week 12 Eating difficulties

Family-based treatment for anorexia nervosa (FBT, "Maudsley") is a first-line intervention for adolescents. It is agnostic about cause and externalises the illness: the family is recruited to defeat the eating disorder, not blamed for it.

Three phases. 1) Parents take charge of refeeding; 2) control is returned to the adolescent, age-appropriately; 3) identity and developmental issues are addressed.

Medical safety. Eating disorders carry the highest mortality of any mental disorder. Weight, vital signs and bloods are medical matters; do not work in isolation.

Avoid. Explaining the illness as a consequence of family dynamics; power struggles over food in session; and colluding with the idea that the young person can simply "decide to eat".

Week 13 Schools and paediatric services

The school as a system. Teachers see the child in a different context and hold different data. Ask for specifics, not impressions: attendance, incidents, academic trajectory, peer relationships, and what strategies have been tried.

Writing a useful report for a school. One page: what you observed, what it means, three concrete recommendations, and who to contact. Avoid diagnosis-led recommendations the school cannot implement.

Paediatric liaison. Distinguish functional from organic symptoms without dismissing either. A referral that names what you want excluded is more likely to be answered (see MFT 707).

> **REPORT**
> Write a one-page school communication for a child with school refusal, including a graded return plan with named adults.

Week 14 Neurodivergence in family context

Reframe. Autism and ADHD are not behavioural choices and not caused by parenting. But the environment determines how disabling they are.

Practical family work.

- Psychoeducation and demystification for siblings and grandparents.
- Environmental adaptation: predictability, visual schedules, sensory accommodations, transition warnings.
- Parental coaching: reduce the demand avoidance cycle; increase specific praise; build in recovery time.
- Couple work: neurodivergence in a partner or child strains the parental alliance; address it explicitly.
- School liaison: accommodations are usually more effective than consequences.

Avoid. Behavioural contracts for skills the child does not yet have; blaming parents; and treating a diagnosis as an explanation for everything.

Week 15 Endings with children

Children end differently. Use a session to review: what we did, what changed, what you will remember, and what you will do when you need help again. Use the medium you used (drawing, play) to close the loop. Give the child something concrete (a certificate, a drawing, a letter) and make sure the carers know when and how to return.

Glossary

Accommodation (of anxiety) family behaviours that reduce a child's distress in the short term and maintain the anxiety. **Assent** a minor's agreement to treatment, alongside guardian consent. **Externalising (the illness)** separating the child from the disorder, as in FBT. **Family-based treatment (FBT)** a manualised family approach to adolescent anorexia nervosa. **Parent management training (PMT)** behavioural skills training for parents of children with conduct problems. **School refusal** difficulty attending school owing to emotional distress, distinct from truancy. **Separation anxiety** developmentally excessive distress about separation from attachment figures.

Self-assessment

Multiple choice

1. The strongest evidence for child conduct problems is for: (a) individual play therapy (b) parent management training (c) family sculpture (d) medication only

2. In anxiety work, family accommodation: (a) is helpful (b) maintains anxiety despite reducing short-term distress (c) is irrelevant (d) should be punished

3. Asking an adolescent directly about suicide: (a) increases risk (b) does not increase risk and is required (c) should be avoided (d) requires parental consent

4. FBT for anorexia: (a) blames parents (b) recruits parents to defeat the illness (c) focuses on insight only (d) is individual therapy

5. School refusal differs from truancy in that it involves: (a) deception without distress (b) emotional distress about attending (c) peer pressure (d) academic failure

6. A child's disclosure of abuse should be: (a) kept confidential (b) documented verbatim and reported (c) discussed with the alleged abuser (d) ignored if vague

7. The strongest predictor of recovery after maltreatment is: (a) the child's temperament (b) a protective, believing caregiver (c) the severity of abuse (d) therapy length

8. Neurodivergence is best addressed by: (a) behavioural contracts (b) environmental adaptation and skills-based support (c) blaming parents (d) exclusion

9. Divided loyalty refers to: (a) the therapist's loyalty to the agency (b) the child's conflict between therapist and parent (c) sibling rivalry (d) custody disputes

10. Telling children about divorce should: (a) include details of the affair (b) be done by both parents together without blame (c) be left to the courts (d) wait until they ask

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. List six red flags requiring referral in child assessment.

2. Describe the three drawing/play techniques and what each reveals.
3. How would you explain confidentiality to a 14-year-old?
4. Give the five core skills of parent management training.
5. What are the three phases of FBT and what happens in each?

Essay (choose one, 2,000 words)

1. "There is no such thing as child therapy without the family." Discuss with evidence.
2. Using a case, show how a developmental formulation changed the treatment of a behaviour problem.
3. Analyse the ethical tensions in working with a 15-year-old whose parents disagree with her treatment goals.

Further reading

- Carr, A. (2019). Family Therapy: Concepts, Process and Practice (3rd ed.). Wiley.
- Kazdin, A. E. (2018). Parent Management Training. OUP.
- Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford.
- Sroufe, L. A. et al. (2005). The Development of the Person. Guilford.
- Siegel, D. J., & Bryson, T. P. (2011). The Whole-Brain Child. Delacorte.

Next: MFT 712 Crisis Intervention Strategies

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 12 OF 28

MFT 712

Crisis Intervention Strategies

15 teaching weeks 3 practicals 2 figures

SEMESTER TWO

1.71 units Prerequisite: MFT 704

> A crisis is not a bigger version of an ordinary session.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 712 Crisis Intervention Strategies

MSc Marriage and Family Therapy Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 704

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 712 Crisis Intervention Strategies. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	**Crisis Intervention Strategies**	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 What makes a crisis a crisis . . . 6

Week 2 The first ten minutes . . . 7

Week 3 Crisis assessment and triage . . . 7

Week 4 Suicide I: assessment . . . 8

Week 5 Suicide II: safety planning . . . 9

Week 6 Simulated crisis assessment . . . 10

Week 7 Self-harm and repeated crisis . . . 10

Week 8 Domestic violence and danger assessment . . . 10

Week 9 Child protection emergencies . . . 11

Week 10 Acute grief and bereavement . . . 11

Week 11 Family crises . . . 12

Week 12 Disasters and collective trauma . . . 13

Week 13 Telephone, text and remote crisis work . . . 13

Week 14 The crisis team and continuity . . . 14

Week 15 Vicarious trauma and sustainable practice . . . 14

Glossary . . . 14

Self-assessment . . . 14

- Multiple choice . . . 14
- Answer key . . . 15
- Short answer . . . 15
- Essay (choose one, 2,000 words) . . . 15

Further reading . . . 16

Course leader's summary. A crisis is not a bigger version of an ordinary session. It is a different clinical state with its own rules: safety before insight, structure before exploration, and short contacts with clear follow-up. This course will make you useful in the first hour.

How to use this book

The course is built around the **first hour**: what you do in the ten minutes after a disclosure of suicide, a violent incident, a sudden death, or a family that arrives in chaos. Read the scripts aloud. Under stress, you will use the sentences you have rehearsed, not the theory you remember.

1. Course description

Crises are the point at which families enter services most often and at which therapists feel least prepared. This course covers: crisis theory and the phases of a crisis; assessment of lethality and danger; suicide and self-harm; domestic violence; child protection emergencies; acute grief and bereavement; family crises (separation, disclosure, runaway, arrest); disasters and collective trauma; telephone and remote crisis work; and the organisational context of crisis services.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess a crisis using a structured model, and prioritise safety	Observed assessment
LO2	Conduct a suicide risk assessment and co-create a safety plan	Skills test
LO3	Manage acute family conflict and violence safely in session	Simulation
LO4	Respond to acute grief, shock and disorganisation	Case study
LO5	Work within the limits of a single session and a crisis team	Viva
LO6	Document crisis work defensibly	Portfolio
LO7	Protect yourself from vicarious trauma in crisis work	Reflective portfolio

3. Assessment

Component	Weight	Due
Simulated crisis assessment (30 min)	30%	Week 6
Safety plan portfolio (3 plans with rationale)	25%	Week 10
Written examination (case-based)	25%	Week 16
Crisis log and reflection (10 entries)	20%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What makes a crisis a crisis	Map a crisis in phases
2	The first ten minutes: safety, contact, structure	Opening drill
3	Crisis assessment and triage	Triage exercise
4	Suicide I: assessment	Observed risk interview
5	Suicide II: safety planning and means restriction	Safety plan lab
6	**Simulated crisis assessment**	Assessed
7	Self-harm and repeated crisis	Containment planning
8	Domestic violence and danger assessment	Safety planning with a victim
9	Child protection emergencies	Reporting rehearsal
10	**Safety plan portfolio due**; acute grief and bereavement	Death notification role-play
11	Family crises: runaway, arrest, disclosure, eviction	Multi-agency simulation
12	Disasters and collective trauma	PFA demonstration
13	Telephone, text and remote crisis work	Call simulation
14	The crisis team, handover and follow-up	Handover practice
15	Vicarious trauma and sustainable crisis work	Personal plan

Week 1 What makes a crisis a crisis

Definition. A crisis is a state of psychological disequilibrium in which a person's usual coping methods fail and their functioning is impaired, usually following a precipitating event that is perceived as a threat. It is **time-limited** (typically 4 6 weeks), **dangerous** (risk of harm), and **an opportunity** (people are more open to help than at any other time).

The four phases (Caplan).

1. **Impact / shock** disorganisation, disbelief, numbness.
2. **Tension and disorganisation** attempts to cope fail, distress rises, help-seeking begins.
3. **Mobilisation** active attempts to solve, often chaotic.
4. **Resolution or breakdown** adaptation, or chronic dysfunction.

The precipitant is not the whole story. A crisis occurs when a stressor meets a vulnerability. Two families can experience the same event and only one present in crisis. Assess both.

Family crisis is systemic. One member's crisis reorganises the household: roles shift, secrets surface, and previously stable patterns break. Your assessment is of the family's equilibrium, not only the identified person's.

> **PRACTICAL**
> Take a crisis you have witnessed (disguised). Map it onto the four phases and identify where intervention would have changed the trajectory.

Week 2 The first ten minutes

Crisis work has a protocol. In the first ten minutes you do five things, in this order:

1. **Safety.** Is anyone in immediate danger? Are you safe? Is there a weapon, a person at the door, an overdose already taken?
2. **Contact.** Your voice, your pace, your presence. Lower your tone, slow your speech, sit down, say your name and your role.
3. **Structure.** Tell them what is going to happen in the next hour. "I am going to ask you some difficult questions. Then we will make a plan together."
4. **Story.** Let them tell it, but with a boundary: "Start from the beginning; we have twenty minutes."
5. **Mobilise help.** Who is available? Who can come? What has helped before?

Sentences that work.

- "I am glad you told me. You are not in trouble."
- "I need to ask you some direct questions. They may feel intrusive. I am asking because I need to know how to keep you safe."
- "You do not have to solve this tonight. We need to get through the next twenty-four hours."

Sentences that do not work. "Everything will be fine." "You should not feel that way." "At least " "Do you really mean that?"

Week 3 Crisis assessment and triage

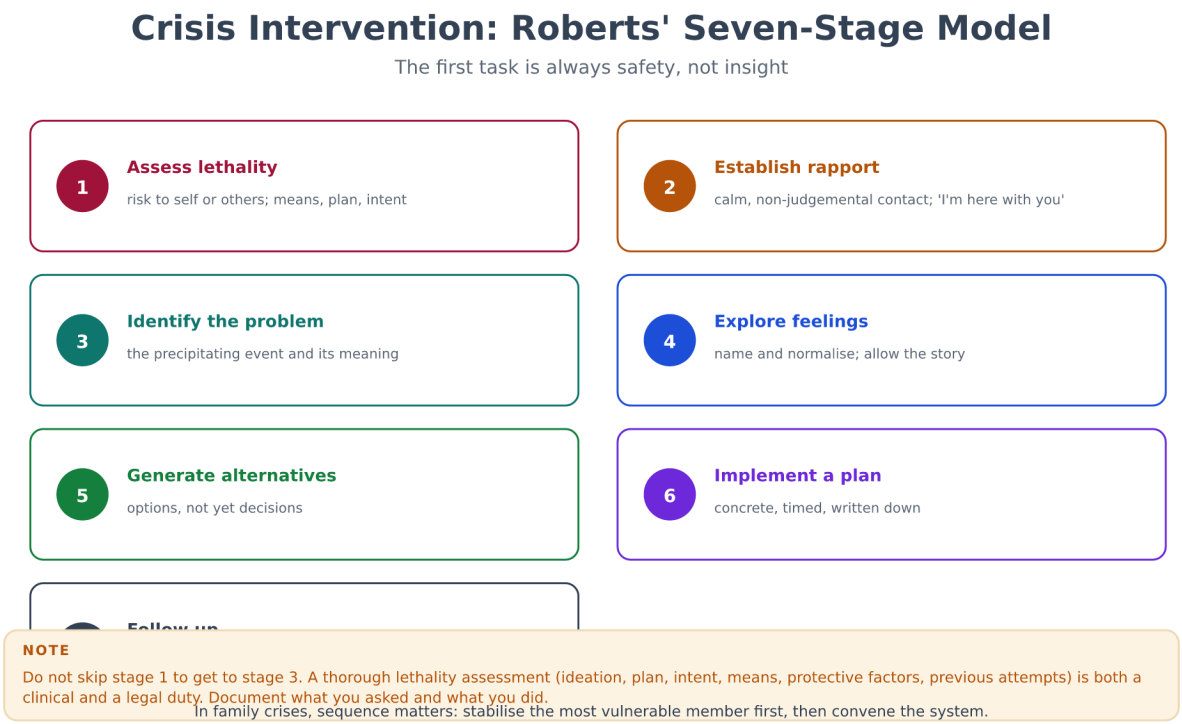


Figure 3.1 Roberts' seven-stage crisis intervention model

The seven stages (Roberts) provide a structure that survives chaos: assess lethality establish rapport identify the problem and precipitant explore feelings generate alternatives implement a plan follow up.

Triage: three questions.

1. **Is there immediate danger?** (Weapon, overdose, assault in progress, child alone with a dangerous adult.)
2. **What is the appropriate level of care?** (Emergency department, crisis team, same-week appointment, routine.)
3. **Who else needs to know?** (Partner, school, social services, police, GP.)

Do not provide long-term therapy in a crisis. Your task is stabilisation, safety and connection to ongoing care. A crisis service that becomes the client's therapy creates dependence and denies the client the long-term work.

Week 4 Suicide I: assessment

The epidemiology you need. Suicide is among the leading causes of death in young people; men die more often, women attempt more often; previous attempt is the strongest predictor; access to means matters; most people who die by suicide have communicated something to someone.

The structured assessment (ten domains). Ideation, plan, intent, means, previous attempts, protective factors, substance use, impulsivity, family history, current stressors plus a direct question about what has kept you alive so far.

Asking.

> "When you say you cannot go on, I need to be clear: are you thinking about ending your life?" "Have you thought about how you would do it?" "Do you have [the means] at home?" "Have you ever tried before?" "What has kept you going until now?"

Formulation and risk level.

Level	Picture	Action
Low	Ideation without plan, strong protective factors, engaged	Safety plan, follow-up within 1 week, means counselling
Moderate	Ideation with plan, limited intent, some protective factors	Same-day mental health review, means restriction, daily contact
High	Plan, intent, means, recent attempt, agitation, intoxication	Do not leave alone; emergency department or crisis team; consider involuntary route

Documentation. Record the questions and answers verbatim where possible, the risk level, the rationale, the plan, who was informed, and the review. If you judged risk lower than others might, state why.

Week 5 Suicide II: safety planning

A safety plan is not a no-suicide contract. Contracts have no evidence base and can increase guilt. A safety plan is a written, collaborative, stepwise coping plan.

Six components (Stanley & Brown).

1. **Warning signs** the person's own early signals ("I stop answering my phone").
2. **Internal coping** things they can do alone ("walk to the mosque", "cold shower", "music").
3. **People and places that distract** social settings that reduce isolation.
4. **People to ask for help** names and numbers.
5. **Professionals and agencies** crisis line, clinic, emergency number.
6. **Making the environment safe** means restriction: who holds the medication, the keys, the pesticide.

Means restriction is the most effective single intervention. Ask directly: "What is in the house?" Ask the family to take control of it, and be specific about who, what, and where.

With families. The family's response matters: reduce blame, increase supervision without surveillance, and give the person a role in their own plan.

PRACTICAL

Co-create a safety plan with a role-played client. The assessor checks: was it collaborative? Were means addressed specifically? Was a follow-up date agreed?

Week 6 Simulated crisis assessment

Thirty minutes, unannounced scenario (suicide risk, domestic violence, or acute grief). Assessed on: safety first, contact and pace, structured assessment, formulation, plan, and documentation quality.

Week 7 Self-harm and repeated crisis

Self-harm is often a regulation strategy, not a suicide attempt. Distinguish: function (relief, control, punishment, communication, feeling something), lethality, frequency, and hidden versus disclosed.

Working with repeated crises. Frequent attenders are often described as "manipulative" a term that should be banned. The pattern usually reflects: an untreated disorder, chronic trauma, an unstable relationship system, or a service that rewards crisis and does not provide continuity.

The intervention is a plan, not a lecture. A written crisis protocol agreed with the person (what to do, who to call, when to come to hospital) reduces both crisis frequency and staff burnout.

Therapist boundaries. You cannot be available 24 hours. State your availability clearly, give the crisis route for out-of-hours, and do not give your personal number unless your service requires it and you have thought through the consequences.

Week 8 Domestic violence and danger assessment

Danger assessment questions (adapted from the Danger Assessment).

- Has violence increased in frequency or severity in the last year?
- Has the partner ever used a weapon or threatened one?
- Has the partner ever tried to strangle or choke you?
- Has the partner threatened to kill you or the children?
- Is the partner violently or constantly jealous and controlling?
- Is there a separation in progress (the highest-risk period)?
- Does the partner have access to firearms?
- Has the partner ever forced sex?
- Is the partner using substances heavily?
- Has the partner threatened suicide?

Safety planning with a victim. Identify a safe place, a code word, documents and money, a bag prepared, children's needs, transport, and the number of a specialist service. Do not tell her to leave as though leaving were simple; leaving is the most dangerous period, and she is the expert on her own risk.

Do not conduct conjoint sessions where coercive control is present. Do not inform the abuser of the disclosure. Know your mandatory reporting obligations and their limits (in many jurisdictions, adult IPV is not mandatorily reportable, but child exposure may be).

Week 9 Child protection emergencies

Signs requiring immediate action.

- A disclosure of abuse (physical, sexual, emotional, neglect).
- Unexplained injury inconsistent with the explanation.
- A child who is frightened of a specific adult.
- A child left alone or in the care of someone unable to protect them.
- Sexualised behaviour beyond developmental norms.
- A parent who cannot or will not protect the child.

Procedure.

1. Respond calmly; do not interview in detail; use open prompts only ("tell me what happened").
2. Record verbatim, promptly, with the child's words and your questions.
3. Do not promise secrecy.
4. Report to the designated safeguarding lead or the appropriate authority, following your service's procedure and your jurisdiction's law (in Nigeria, the Child Rights Act and state child protection structures; the police family unit; the state ministry responsible for social welfare).
5. Tell the child what will happen next and who will be involved.
6. Document everything, including the advice you received and the time you acted.

> **REHEARSAL**
> Practise the first four minutes of a disclosure, and the phone call to report it.

Week 10 Acute grief and bereavement

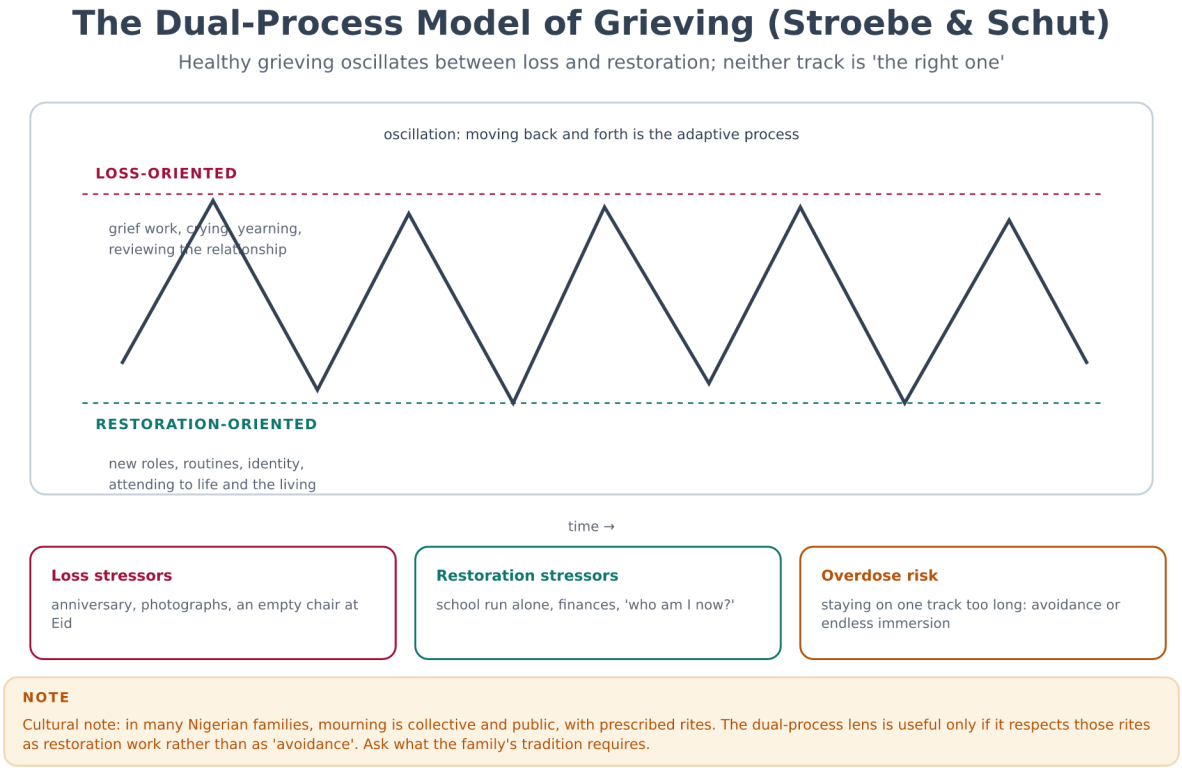


Figure 10.1 The dual-process model of grieving

Normal acute grief includes waves of yearning, disbelief, anger, guilt, intrusive images, and a sense of unreality. It is not a disorder. It becomes complicated when it is prolonged, severe, and impairing (prolonged grief disorder in ICD-11/DSM-5-TR).

In the first weeks: accompany, do not treat. Practical help, information, permission to grieve, and continuity. Do not rush to meaning-making; do not say "he is in a better place" unless you know the family's faith.

Sudden and traumatic deaths (accident, homicide, suicide) add shock, anger, stigma and, for suicide, a family burden of blame and secrecy. Address the stigma explicitly.

Children and grief. Children grieve in bursts, return to play, and ask direct questions. Be honest, use clear language ("died", not "lost"), and reassure them that they are not to blame.

The dual-process frame. Healthy grieving oscillates between loss-oriented and restoration-oriented activity. Two errors: staying only on the loss track (rumination, avoidance of life) and staying only on the restoration track (avoidance of grief). The family's culture prescribes which is expected; work within it.

Week 11 Family crises

Crisis	Immediate priorities	Common errors
Adolescent runaway	Safety, whereabouts, who they trust, why they left	Interrogating the parents first; treating the adolescent as the problem
Arrest or imprisonment	Practical information, children's needs, shame, finances	Moralising; ignoring the children
Disclosure of a secret (affair, paternity, HIV, debt)	Containment, safety, who needs to know and when	Forcing immediate disclosure; colluding in concealment
Eviction or financial collapse	Practical referral; safety; children's schooling	Psychologising material reality
Sudden separation	Safety, children's stability, legal referral	Assuming reconciliation is the goal
Death of a child	Presence, not technique; couple's survival	Interpreting; taking sides in blame

Week 12 Disasters and collective trauma

Psychological first aid (PFA) five elements: safety, stabilisation, connection, self-efficacy, hope. It is not debriefing. Mandatory psychological debriefing has been shown to be ineffective and possibly harmful; do not do it.

Principles in collective events.

- Attend to basic needs first (shelter, food, information, family reunification).
- Do not pathologise normal distress.
- Reconnect people to their own networks and rituals; community and faith resources are usually more powerful than clinical intervention in the acute phase.
- Identify the vulnerable: unaccompanied children, the injured, the bereaved, those with prior trauma, and responders.
- Plan for the medium term: most distress resolves; a minority need treatment at 4 12 weeks.

Week 13 Telephone, text and remote crisis work

Advantages. Access, immediacy, anonymity, reach into areas with no services.

Risks. Loss of non-verbal information, difficulty verifying location, inability to intervene physically, and boundary erosion.

Protocol for a remote crisis contact.

1. Verify identity and location at the start ("where are you right now?").
2. Establish whether anyone is with them.
3. Assess risk verbally, more slowly than you would in person.

4. Agree a concrete plan for the next hour.
5. Record the number they called from and a safe contact.
6. If risk is high, keep them on the line while arranging help.

Week 14 The crisis team and continuity

Handover. A good handover is structured: presenting problem, risk level and basis, what has been done, what is planned, who is responsible, and when the next contact is. Use a standard format (e.g. SBAR: Situation, Background, Assessment, Recommendation).

Follow-up is the intervention. Crisis care that does not follow up fails. Agree a date before the person leaves, and make the contact proactive.

Working with other agencies. You will share information; know the legal basis, share the minimum necessary, and record what you shared and why.

Week 15 Vicarious trauma and sustainable practice

Crisis work takes a physiological toll. Warning signs: intrusive images, sleep disruption, cynicism, numbness, dread before shifts, irritability at home, and loss of pleasure in work.

Protective practices. Debrief structurally (not emotionally) after critical incidents; use supervision; keep caseload diversity; maintain boundaries on availability; sleep; exercise; and have your own therapist.

The organisational dimension. Individual self-care cannot compensate for an unsafe service. If you are carrying 40 crisis cases with no supervision, that is a management problem, and saying so is part of professional practice.

Glossary

Containment the provision of structure and safety so that overwhelming material can be held. **Crisis** a time-limited state of disequilibrium in which usual coping fails. **Danger assessment** structured assessment of lethality risk in domestic violence. **Means restriction** reducing access to methods of self-harm; the most effective suicide prevention measure. **Psychological first aid (PFA)** a five-element supportive intervention after a critical event. **Safety plan** a written, collaborative, stepwise plan for managing a suicidal crisis. **Triage** the prioritisation of care according to urgency and risk.

Self-assessment

Multiple choice

1. The first task in a crisis is: (a) insight (b) safety (c) diagnosis (d) history

2. Roberts' model begins with: (a) rapport (b) lethality assessment (c) plan (d) follow-up

3. A no-suicide contract is: (a) evidence-based (b) not evidence-based; a safety plan is preferred (c) legally required (d) sufficient alone

4. The highest-risk period in domestic violence is: (a) during pregnancy (b) separation (c) the first year (d) after counselling

5. Psychological debriefing after disasters is: (a) recommended for all (b) not recommended and potentially harmful (c) legally required (d) equivalent to PFA

6. Means restriction is important because: (a) it is easy (b) it reduces the probability of a lethal attempt (c) it is legally required (d) it satisfies families

7. Conjoint therapy is contraindicated when: (a) there is conflict (b) there is coercive control (c) there is grief (d) there is low income

8. A child's disclosure of abuse should first be: (a) investigated by you (b) recorded verbatim and reported (c) discussed with the parents (d) kept confidential

9. Prolonged grief disorder is distinguished from normal grief by: (a) crying (b) duration, severity and impairment (c) cultural expression (d) family conflict

10. A good crisis handover includes: (a) only the diagnosis (b) situation, background, assessment, recommendation, and next contact (c) your opinion of the family (d) the full case notes

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. List the five things you do in the first ten minutes of a crisis.
2. Give six components of a safety plan.
3. Name three questions from a danger assessment.
4. What are the five elements of psychological first aid?
5. Why is follow-up the intervention in crisis work?

Essay (choose one, 2,000 words)

1. "Crisis is both danger and opportunity." Discuss with a case, including what you did and what you would change.
2. Analyse the ethical tensions in responding to a suicidal adolescent whose parents refuse hospital admission.
3. Critically evaluate the evidence on psychological debriefing and its implications for crisis services.

Further reading

- Roberts, A. R. (2005). Crisis Intervention Handbook (3rd ed.). Oxford University Press.
- James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage.
- Stanley, B., & Brown, G. K. (2012). Safety planning intervention. Cognitive and Behavioral Practice, 19(2), 256 264.
- Hobfoll, S. E. et al. (2007). Five essential elements of immediate and mid-term mass trauma intervention. Psychiatry, 70(4), 283 315.
- Campbell, J. C. (2002). Danger Assessment.

Next: MFT 713 Substance Abuse and Family Dynamics

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 13 OF 28

MFT 713

Substance Abuse and Family Dynamics

15 teaching weeks 4 practicals 1 worked case 2 figures

SEMESTER TWO

1.71 units Prerequisite: MFT 710

> Addiction is a family-organising process.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 713 Substance Abuse and Family Dynamics

MSc Marriage and Family Therapy Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 710

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 713 Substance Abuse and Family Dynamics. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	**Substance Abuse and Family Dynamics**	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 Models of addiction . . . 6

Week 2 The family cycle . . . 7

Week 3 Assessment . . . 8

Week 4 Motivational interviewing I . . . 8

Week 5 Motivational interviewing II . . . 9

Week 6 Stages of change and the family . . . 10

Week 7 Relapse prevention . . . 11

Week 8 Behavioural couples therapy for substance use . . . 11

Week 9 Working with relatives: CRAFT . . . 11

Week 10 Enabling, co-dependency and boundaries . . . 12

Week 11 Dual diagnosis . . . 12

Week 12 Medication-assisted treatment . . . 13

Week 13 Recovery as a family process . . . 13

Week 14 Culture, faith, stigma and community . . . 13

Week 15 Integration . . . 14

Glossary . . . 14

Self-assessment . . . 14

- Multiple choice . . . 14
- Answer key . . . 15
- Short answer . . . 15
- Essay (choose one, 2,000 words) . . . 15

Further reading . . . 15

Course leader's summary. Addiction is a family-organising process. This course teaches you to work with the user, the family, and the household system using motivational, behavioural, systemic and recovery-oriented approaches without moralising, colluding or abandoning hope.

How to use this book

Read the case at the start of each week before the theory. Addiction work punishes abstraction: families are exhausted, professionals are frustrated, and the evidence is often misrepresented by both. Hold two principles throughout: **the person is not the substance**, and **the family is not the cause**.

1. Course description

The course covers: models of addiction (disease, learning, biopsychosocial, systemic); assessment; the family cycle of use, secrecy and shame; motivational interviewing; relapse prevention; behavioural couples therapy for substance use; CRAFT (for concerned relatives); the interface with mental illness ("dual diagnosis"); recovery as a family process; medication-assisted treatment; and policy and cultural context.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain and compare models of addiction, including their clinical implications	Exam
LO2	Conduct a substance-use assessment, including readiness to change	Observed assessment
LO3	Use motivational interviewing skills with fidelity	Skills test
LO4	Formulate the family's role in maintaining and resolving use	Case study
LO5	Deliver CRAFT or behavioural couples work for substance use	Case study
LO6	Work with dual diagnosis and coordinate with medical and recovery services	Viva
LO7	Address stigma, culture and shame in your own practice	Reflective portfolio

3. Assessment

Component	Weight	Due
Motivational interviewing observed (25 min)	25%	Week 5
Family formulation and intervention plan (2,500 words)	35%	Week 11
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio (6 entries)	15%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Models of addiction and their consequences	Model comparison debate
2	The family cycle: use, secrecy, enabling, crisis, shame, promise	Cycle mapping
3	Assessment: quantity, function, consequences, readiness	Structured interview
4	Motivational interviewing I: OARS and the spirit	OARS drill
5	**MI observed**; MI II: change talk and discord	Change-talk practice
6	The stages of change and the family's stage	Stage-matched planning
7	Working with the user: CBT and relapse prevention	Relapse prevention plan
8	Working with the couple: behavioural couples therapy	Daily recovery plan
9	Working with relatives: CRAFT and boundaries	CRAFT role-play
10	Co-dependency, enabling and the limits of the concept	Boundary-setting lab
11	**Case plan due**; dual diagnosis	Integrated plan
12	Medication-assisted treatment and the medical interface	Referral exercise
13	Recovery, relapse and the family's long haul	Relapse response plan
14	Culture, faith, stigma and the community	Community resource map
15	Integration: what works, for whom	Case presentations

Week 1 Models of addiction

Model	Core claim	Clinical implication	Critique
Moral	Use is a choice; user is weak or bad	Punishment, exclusion	Shame increases use; ignores biology
Disease / medical	Addiction is a chronic brain disease	Treatment, medication, compassion	Can reduce agency; "once an addict always" fatalism
Learning / behavioural	Use is reinforced behaviour	Skills, contingency management	Can ignore neurobiology and context
Biopsychosocial	Biology psychology social context	Multimodal treatment	Descriptive rather than prescriptive
Systemic / family	Use is embedded in and organises relationships	Family-inclusive treatment	Risk of blaming the family

Model	Core claim	Clinical implication	Critique
Recovery / social	Recovery is a social process, not only clinical	Peer support, community, identity	Needs availability of recovery capital

The position used here. Addiction is a **chronic, relapsing condition** shaped by biology, learning, trauma, relationship, culture and economics. The person retains agency but not unlimited agency. Families are neither causes nor cures; they are the environment in which recovery either becomes possible or does not.

> **DEBATE**
> "Addiction is a disease" versus "addiction is a choice". Take a side, then argue the other side, then write the practical position you will hold with clients.

Week 2 The family cycle

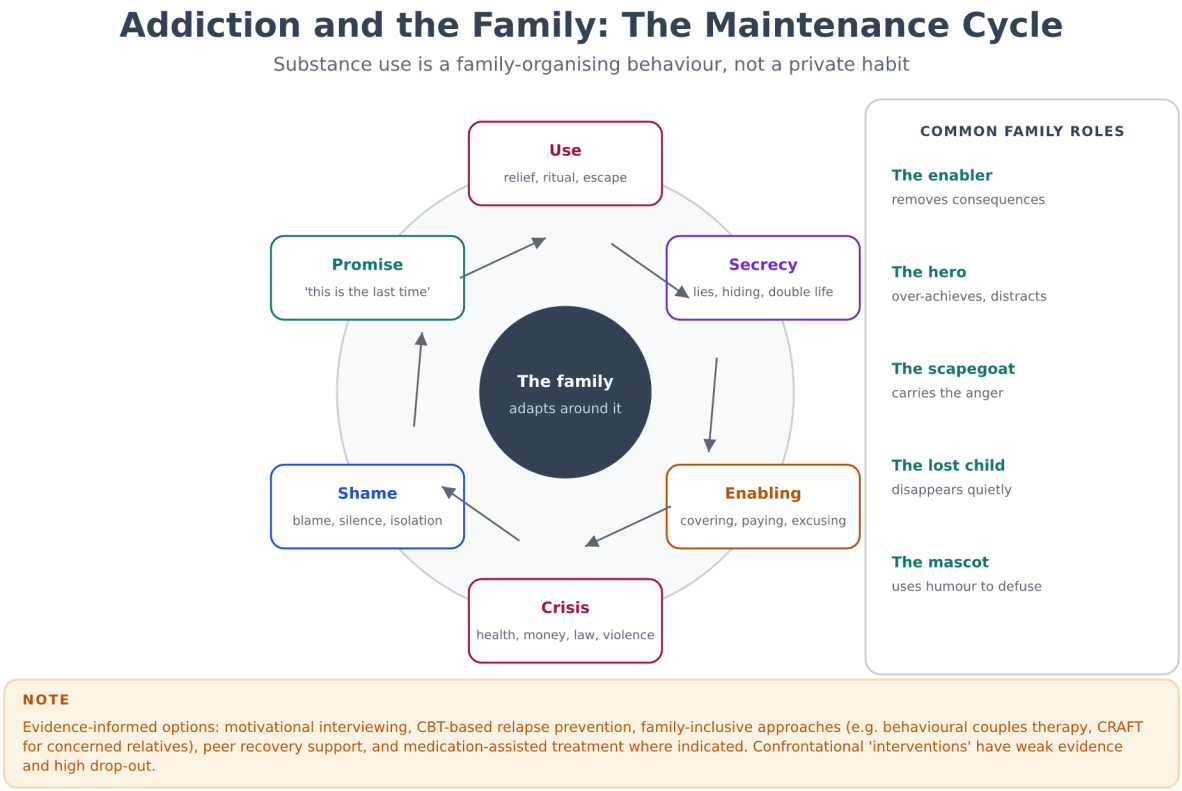


Figure 2.1 The addiction and family maintenance cycle

Six stations that repeat: use secrecy enabling crisis shame promise use.

Enabling is not weakness. It is a set of behaviours that reduce immediate consequences because the alternative (the person losing their job, being arrested, or dying) feels unbearable: paying a debt, calling in sick, hiding the bottles, giving money "for food".

Family roles (an imperfect but useful map): the enabler, the hero, the scapegoat, the lost child, the mascot. Use them as questions, never as labels: "Who in the family keeps the peace? Who takes the blame? Who disappears?"

The systemic point. The cycle is maintained by both the using and the accommodating. This is not a statement of blame; it is a statement of leverage. Change the accommodating behaviour and the cycle must change.

> **PRACTICAL**
> Map a family cycle from a disguised case. At each station, write: who does what, what they feel, and what they fear. Then identify two points of leverage.

Week 3 Assessment

Substance history.

Domain	Questions
Substances	What, how much, how often, by what route, for how long
Pattern	Time of day, alone or social, binge or daily
Function	What does it do for you? (sleep, courage, relief, numbness, belonging)
Consequences	Health, money, work, relationships, legal, children
Control attempts	Have you tried to stop? What happened? How long?
Withdrawal	Symptoms, previous seizures, previous medically assisted withdrawal
Dependence	Tolerance, withdrawal, compulsion, time spent, continued despite harm
Readiness	Do you want to change? Why now? What would it take?
Risk	Suicide, violence, driving, overdose, child safety

Validated tools. AUDIT (alcohol), DUDIT (drugs), ASSIST (multiple substances), CRAFFT (adolescents). Use them as a structure for conversation, not a verdict.

Assess the family separately. With the user absent: what has been tried, what has been covered up, what each member fears, and who is at risk.

Week 4 Motivational interviewing I

The spirit (Miller & Rollnick): partnership, acceptance, compassion, evocation. You are not the expert on what they should do; you are the expert on a conversation that helps them say it themselves.

The four core skills (OARS).

Skill	What it is	Example
Open question	Invites elaboration	"What is a typical evening like?"
Affirmation	Names a strength, specifically	"You kept going to work through all this."
Reflection	Returns content or feeling	"You are worn out by the effort it takes to keep this hidden."
Summary	Collects and connects	"So on one hand it helps you sleep; on the other it is costing you your marriage."

What to avoid. The righting reflex (the urge to fix), confronting, warning, advising without permission, and arguing. When you argue for change, the client argues against it and you have made change less likely.

Reflection is the active ingredient. Aim for two reflections for every question, and for complex reflections (which add meaning) over simple ones (which repeat content).

> **PRACTICAL**
> OARS drill: 8 minutes, no advice, no questions after the first one. Observer counts reflections, and rates how many were complex.

Week 5 Motivational interviewing II

Change talk client statements in favour of change. Four types: desire ("I want to"), ability ("I could"), reasons ("because "), need ("I have to"). Also mobilising talk: commitment ("I will"), activation ("I am ready"), taking steps ("I bought the medication").

Discord is not resistance; it is a signal about the relationship. It appears as arguing, interrupting, discounting, ignoring. The response is not confrontation but **rolling with it**: "You are not sure this is worth it. Tell me more about that."

The MI-consistent techniques.

- **Elicit provide elicit:** "What do you already know about what alcohol does to the liver?" brief, neutral information "What does that mean for you?"
- **The decisional balance:** "What are the good things about using? And the less good things?" (Ask in that order, always.)
- **Looking forward:** "If nothing changes, where do you imagine you will be in five years?"
- **Readiness ruler:** "On 0 10, how important is it to change? How confident are you that you could? What would move you from 4 to 6?"

Avoid the readiness trap: a score of 3 is an argument for a smaller intervention, not for discharge.

ASSESSED

25-minute MI session with a trained role-player. Rated on: spirit, OARS fidelity, ratio of reflections to questions, and handling of discord.

Week 6 Stages of change and the family

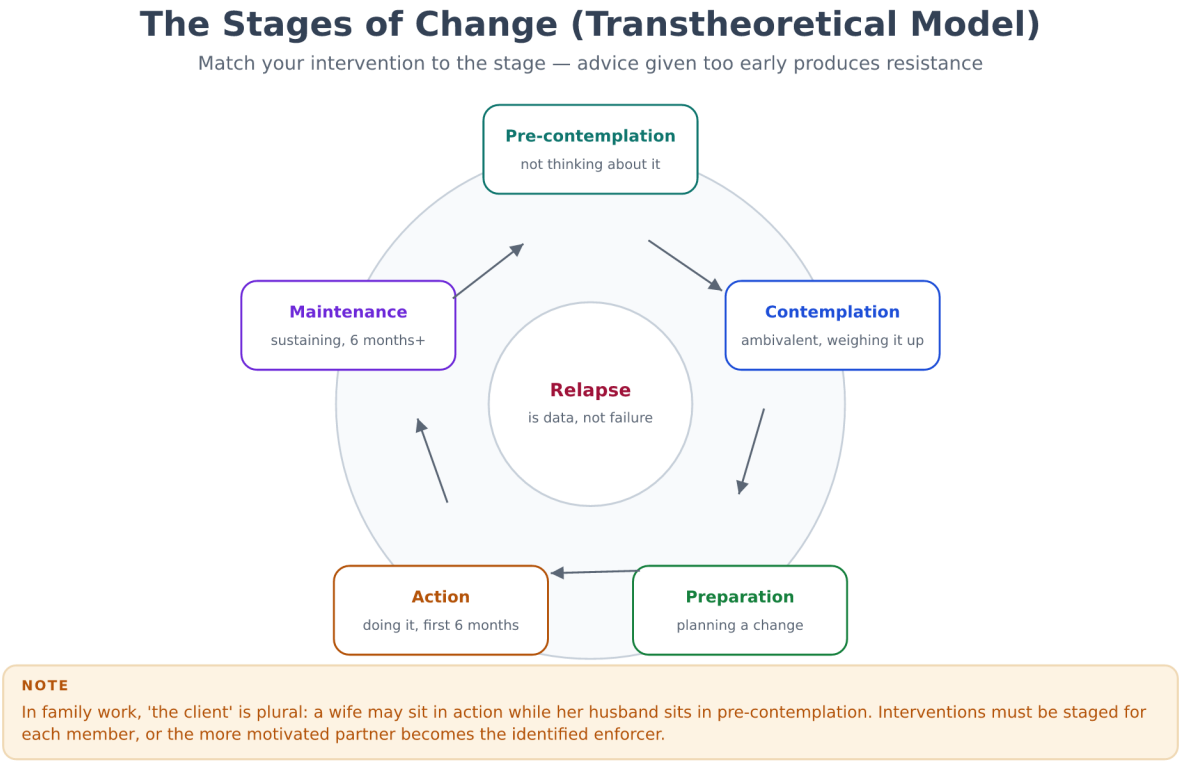


Figure 6.1 The stages of change, including relapse

Stage	The person	The family member	Your task
Pre-contemplation	Not thinking about change	Frustrated, angry, exhausted	Raise doubt gently; do not push
Contemplation	Ambivalent	Hopeful and suspicious	Tip the balance; build confidence
Preparation	Planning	Mobilising	Concrete plan; remove barriers
Action	Changing	Adjusting, testing trust	Support, skills, relapse prevention
Maintenance	Sustaining	Rebuilding	Protect gains; plan for risk
Relapse	Using again	Devastated, "I told you"	Reframe as data; re-plan fast

The family's stage often differs from the user's. A wife in action married to a husband in pre-contemplation becomes the "enforcer", which damages the relationship and does not work. Stage-matched intervention means working separately where necessary.

Week 7 Relapse prevention

The model (Marlatt & Gordon). Relapse is a process, not an event: high-risk situations poor coping lapse the abstinence violation effect ("I've blown it, so I may as well continue") relapse.

The plan.

1. **High-risk situations** people, places, times, feelings (loneliness, anger, celebration, payday).
2. **Coping strategies** for each.
3. **A lapse protocol** what to do in the first 24 hours, and whom to call.
4. **Lifestyle balance** the "shoulds" versus the "wants"; boredom and resentment are relapse risks.
5. **The family's role** agreed, non-punitive responses to a lapse, decided in advance.

With the family. A relapse response agreed in advance prevents the cycle of crisis, shame and secrecy from restarting. Write it down: "If I use again, we will "

Week 8 Behavioural couples therapy for substance use

The evidence. Behavioural couples therapy (BCT) for alcohol and drug problems has good evidence, particularly where the partner is non-using and the relationship is not violent.

The "daily recovery plan" (O'Farrell). Each day: a shared trust discussion in which the abstinent partner states their intention and the other expresses support, then a specified rewarding activity together. It is behavioural, daily, and builds the relationship as a recovery resource.

Why couples work helps. Partners are often the strongest motivator and the most reliable monitoring system but only if the relationship becomes a source of reward rather than a site of surveillance.

Contraindications. Current violence, coercive control, a partner with active untreated addiction, or a relationship the client is being coerced to remain in.

Week 9 Working with relatives: CRAFT

CRAFT (Community Reinforcement and Family Training) works with the concerned significant other to change the contingencies around the user's behaviour, without confrontation. It has better evidence for treatment entry than traditional "interventions" or Al-Anon-style support alone.

Three goals. 1) Improve the relative's own wellbeing; 2) reduce the user's substance use; 3) increase the chance the user enters treatment.

Core skills taught to the relative.

- **Functional analysis:** what triggers use, what consequences follow, what the relative does.
- **Positive communication:** brief, specific, warm.
- **Allow natural consequences:** stop removing consequences where it is safe.
- **Reward non-use:** notice and reinforce sober days.
- **Domestic safety:** never ask a relative to accept violence.
- **Self-care and social support:** the relative's life must not be entirely about the user.
- **Invitation to treatment:** a script, rehearsed.

> **PRACTICAL**
> CRAFT role-play: a mother of a 24-year-old cannabis user. Teach positive communication and one natural consequence. Assessor rates warmth, specificity and safety.

Week 10 Enabling, co-dependency and boundaries

"Co-dependency" is a contested term. Used carefully it describes patterns of over-functioning and self-neglect in relationships with an addicted person. Used carelessly it blames the relative usually a woman for the addiction.

Better language: accommodation and boundary. Ask what the relative does, what it costs them, and what they would like to do differently.

Boundary setting that works. Specific, behavioural, enforceable, and about the relative's own actions: "I will not lend money. If you come home intoxicated, I will sleep in the other room and we will talk in the morning." Not: "You must stop drinking or I will leave." (An unenforceable ultimatum teaches the family that threats are empty.)

Week 11 Dual diagnosis

The overlap is the rule. Anxiety, depression, PTSD, ADHD and psychosis commonly co-occur with substance use. Direction of causality varies; the treatment must be integrated rather than sequential.

Principles.

1. Treat both, at the same time, by the same team where possible.
2. Do not require abstinence before treating mental illness, and do not require remission of mental illness before treating addiction.
3. Be cautious with medication (benzodiazepines, stimulants) and coordinate with the prescriber.
4. Distinguish substance-induced symptoms from independent disorders (timing and persistence).
5. Trauma is common; assume it until assessed.

CASE PLAN (35%)
A family case with dual diagnosis: assessment, formulation, integrated plan, coordination with medical and recovery services, risk plan, and outcome measures.

Week 12 Medication-assisted treatment

Opioid agonist therapy (methadone, buprenorphine) reduces mortality and improves retention; it is evidence-based treatment, not "substituting one drug for another". **Acamprosate** and **naltrexone** reduce alcohol relapse; **disulfiram** works only with supervision. Withdrawal management alone is not treatment.

The family therapist's role. Support adherence, address stigma within the family, challenge the belief that medication is cheating, and coordinate with the prescriber. Do not advise on doses or discontinuation.

Week 13 Recovery as a family process

Recovery capital (Cloud & Granfield): social (relationships, support), physical (health, housing), human (skills, education, health literacy), cultural (values, belonging). Treatment that builds recovery capital predicts better outcomes than treatment that focuses only on symptoms.

The family's long haul. Early recovery is often called "the pink cloud"; the harder work is at six to eighteen months, when the family must renegotiate roles that were organised around the addiction. A partner who has run everything may not want to hand back control; a child who has been the carer may not want to be a child. Name this, and plan for it.

Relapse is data. The question is not "why did you fail?" but "what happened, and what does the plan need?"

Week 14 Culture, faith, stigma and community

- **Substance use in context:** alcohol is culturally embedded in many Nigerian settings (burial rites, festivals, business); kola, cannabis, tramadol and codeine have specific local patterns.
- **Faith:** deliverance and prayer can be powerful resources and can also delay medical care. Coordinate, do not compete.
- **Stigma:** "a disgrace to the family" keeps use hidden and prevents early help. Your language matters: "person with a substance use disorder", not "addict".
- **Community resources:** mutual aid (AA/NA and their local equivalents), faith-based recovery groups, community health workers map what actually exists near your clinic.

Week 15 Integration

What works: screening and brief intervention; MI; CBT-based relapse prevention; BCT and CRAFT; medication where indicated; integrated treatment for dual diagnosis; recovery support; and continuity over time. What does not: confrontation, "tough love" as a single strategy, sequential treatment for dual diagnosis, and asking a family to do the work alone.

Glossary

Abstinence violation effect the spiral of guilt and continued use after a lapse. **CRAFT** Community Reinforcement and Family Training, for concerned significant others. **Change talk** client statements favouring change. **Dual diagnosis** co-occurring substance use and mental disorder. **Enabling** behaviours that remove the natural consequences of use. **Harm reduction** interventions that reduce harm without requiring abstinence. **Motivational interviewing** a collaborative, directive method for strengthening intrinsic motivation. **Recovery capital** the resources a person can draw on to sustain recovery. **Relapse** return to problematic use after a period of change.

Self-assessment

Multiple choice

1. The righting reflex refers to: (a) the client's resistance (b) the clinician's urge to fix (c) a CBT technique (d) a legal duty

2. CRAFT is aimed primarily at: (a) the user (b) concerned relatives (c) employers (d) courts

3. Change talk includes: (a) sustain talk (b) desire, ability, reasons, need (c) only commitment (d) only questions

4. Medication-assisted treatment is: (a) substituting one drug for another (b) evidence-based treatment that reduces mortality (c) only for the wealthy (d) discouraged

5. The "abstinence violation effect" describes: (a) legal consequences (b) the spiral after a lapse (c) withdrawal (d) family conflict

6. A well-formed boundary is: (a) an unenforceable ultimatum (b) specific, behavioural and about the relative's own actions (c) a threat (d) a diagnosis

7. In dual diagnosis, treatment should be: (a) sequential (b) integrated (c) only psychiatric (d) only addiction-focused

8. Enabling is best understood as: (a) malice (b) reducing immediate consequences out of care (c) a diagnosis (d) irrelevant

9. Relapse should be treated as: (a) failure (b) data for re-planning (c) grounds for discharge (d) proof of character

10. Recovery capital includes: (a) only abstinence (b) social, physical, human and cultural resources (c) only income (d) only family support

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Compare the disease and systemic models of addiction and name one clinical implication of each.
2. Map the six stations of the family cycle and give one intervention at each.
3. What are OARS, and what is the recommended ratio of reflections to questions?
4. Describe the daily recovery plan in behavioural couples therapy.
5. Name the three goals of CRAFT.

Essay (choose one, 2,000 words)

1. "The family is neither the cause nor the cure, but it is the context." Discuss with a case.
2. Critically evaluate the concept of co-dependency and its gendered assumptions.
3. Design a family-inclusive service for adolescents with substance use problems in a Nigerian city.

Further reading

- Miller, W. R., & Rollnick, S. (2013). Motivational Interviewing (3rd ed.). Guilford.
- O'Farrell, T. J., & Fals-Stewart, W. (2006). Behavioral Couples Therapy for Alcoholism and Drug Abuse. Guilford.
- Smith, J. E., & Meyers, R. J. (2004). Motivating Substance Abusers to Enter Treatment: Working with Family Members. Guilford.
- Marlatt, G. A., & Donovan, D. M. (2005). Relapse Prevention (2nd ed.). Guilford.
- Cloud, W., & Granfield, R. (2008). Conceptualizing recovery capital. Substance Use & Misuse, 43(12 13).

Next: MFT 714 Trauma-Informed Care

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 14 OF 28

MFT 714

Trauma-Informed Care

15 teaching weeks 1 practical 5 figures

SEMESTER TWO

1.71 units Prerequisite: MFT 712

> Trauma changes the body, the memory, the attachment system and the family.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 714 Trauma-Informed Care

MSc Marriage and Family Therapy Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 712

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 714 Trauma-Informed Care. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	**Trauma-Informed Care**	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 What trauma is . . . 6

Week 2 The ACE evidence . . . 7

Week 3 Neurobiology I: the window of tolerance . . . 7

Week 4 Neurobiology II: memory, dissociation, the body . . . 9

Week 5 Attachment and developmental trauma . . . 10

Week 6 Trauma-informed assessment . . . 11

Week 7 Observed trauma-informed assessment . . . 12

Week 8 The phased model . . . 12

Week 9 Trauma-focused CBT with children . . . 12

Week 10 Other trauma-specific modalities . . . 13

Week 11 Couples and trauma . . . 13

Week 12 Intergenerational transmission . . . 14

Week 13 Re-traumatisation in services . . . 15

Week 14 Vicarious trauma . . . 15

Week 15 What trauma-informed care is not . . . 15

Glossary . . . 16

Self-assessment . . . 16

- Multiple choice . . . 16
- Answer key . . . 16
- Short answer . . . 17
- Essay (choose one, 2,000 words) . . . 17

Further reading . . . 17

Course leader's summary. Trauma changes the body, the memory, the attachment system and the family. This course teaches the physiology, the phased treatment models, and the organisational stance and it teaches you to avoid the two commonest errors: doing too much too soon, and doing nothing at all.

How to use this book

This is the most personally demanding book in the programme. Announce it in advance to your peers; know your own history; use supervision; and do not read the trauma chapters late at night. The material is presented in the order treatment unfolds: safety, then processing, then integration.

1. Course description

Trauma-informed care means organising practice around the prevalence and impact of trauma; **trauma- specific** care means delivering interventions designed to treat it. You need both.

The course covers: definitions and types of trauma; the Adverse Childhood Experiences evidence; the neurobiology of threat and regulation; dissociation; attachment and developmental trauma; the phased model; specific modalities (TF-CBT, EMDR, narrative exposure, somatic approaches); family and couple work with trauma; intergenerational transmission; re-traumatisation in services; and practitioner self-protection.

2. Learning outcomes

#	Outcome	Assessed
LO1	Define trauma and distinguish types, including complex and developmental trauma	Exam
LO2	Explain the neurobiology of threat, regulation and memory	Exam
LO3	Conduct a trauma-informed assessment without re-traumatising	Observed assessment
LO4	Apply a phased treatment model and justify the sequencing	Case study
LO5	Use at least two trauma-specific interventions under supervision	Skills test
LO6	Work systemically with trauma in couples and families	Case study
LO7	Design trauma-informed organisational practices	Report

3. Assessment

Component	Weight	Due
Trauma-informed assessment observed (40 min)	25%	Week 7

Component	Weight	Due
Phased treatment plan (2,500 words)	35%	Week 12
Trauma-informed service audit report (2,000 words)	20%	Week 14
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	What trauma is: single, complex, developmental, collective	Typology exercise
2	The ACE evidence and its limits	ACE discussion (with care)
3	Neurobiology I: threat, arousal and the window of tolerance	Regulation skills lab
4	Neurobiology II: memory, dissociation and the body	Grounding practice
5	Attachment and developmental trauma	Attachment-informed formulation
6	Trauma-informed assessment	Structured interview
7	**Observed assessment**	Assessed
8	The phased model: safety, processing, integration	Phase planning
9	Trauma-focused CBT with children and families	TF-CBT component practice
10	EMDR, narrative exposure and somatic approaches	Comparison and critique
11	Couples and trauma: betrayal, violence, and the repair of trust	Couple session design
12	**Treatment plan due**; intergenerational transmission	Genogram of trauma
13	Re-traumatisation in services; the four R's	Service audit
14	**Service audit due**; vicarious trauma and supervision	Personal resilience plan
15	Integration: what trauma-informed care is not	Case presentations

Week 1 What trauma is

Definition (DSM-5/ICD-11, paraphrased). Exposure to actual or threatened death, serious injury or sexual violence, whether experienced directly, witnessed, learned about in relation to a close person, or through repeated occupational exposure.

Types.

Type	Description	Example
Single-incident	One event, adult onset	Road traffic collision, armed robbery
Complex	Repeated, prolonged, interpersonal, inescapable	Domestic abuse, childhood abuse

Type	Description	Example
Developmental	Occurs during maturation; shapes personality and attachment	Chronic neglect, chaotic caregiving
Collective	Shared by a community	War, displacement, massacre, epidemic
Vicarious / secondary	Exposure through another's trauma	Therapist, first responder, family carer

What makes an event traumatic is not only the event. It is the event plus the meaning, the interpersonal context (was it caused by someone who was supposed to protect you?), the aftermath (was the person believed? supported?), and prior vulnerability.

Complex PTSD (ICD-11) comprises three additional clusters beyond classic PTSD: affect dysregulation, negative self-concept, and disturbances in relationships. This matters clinically: classic PTSD responds well to exposure-based treatment; complex PTSD usually requires a longer stabilisation phase.

> **PRACTICAL**
> Classify five disguised cases by type, and note which additional features each has.

Week 2 The ACE evidence

The original study. Adverse Childhood Experiences (abuse, neglect, household dysfunction) were found to have a graded relationship with later health outcomes, including depression, addiction, suicide attempts, heart disease and early death.

What it established. Early adversity is common, cumulative, and biologically embedded. It shifted the field from asking "what is wrong with you?" to "what happened to you?"

What it does not establish. That ACE scores are destiny; that any individual's future is determined; or that a high score is a diagnosis. Roughly a third of people with high ACE scores have no significant impairment protective relationships matter enormously.

The clinical use. ACE enquiry is a conversation, not a score: "Was there anything difficult in your childhood that we should know about? You do not have to give details."

> **CAUTION**
> Asking about adversity can activate distress. Ask when you have time to respond, offer a choice, and have a grounding plan ready.

Week 3 Neurobiology I: the window of tolerance

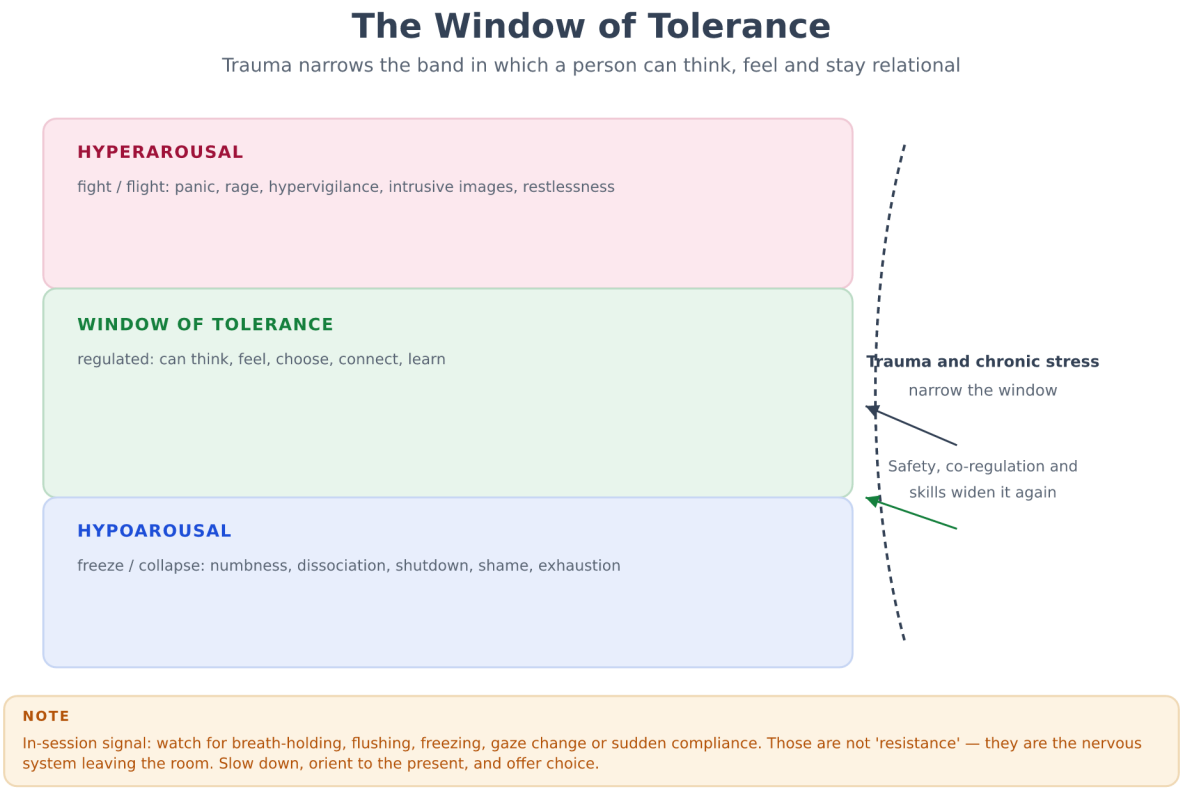


Figure 3.1 The window of tolerance

The three states.

- **Within the window:** the person can think, feel, choose, connect and learn.
- **Hyperarousal:** fight/flight panic, rage, hypervigilance, intrusive images, restlessness.
- **Hypoarousal:** freeze/collapse numbness, dissociation, shutdown, shame, exhaustion.

Trauma narrows the window. Small triggers produce large reactions; the person's capacity to stay relational shrinks. This is not "over-reacting"; it is a nervous system doing what it learned.

Regulation skills to teach (and use in session).

1. **Paced breathing** exhale longer than inhale (4 in, 6 out) for 90 seconds.
2. **Orienting** look around the room and name five things you can see.
3. **Grounding** feet on the floor, weight in the chair, describe the texture of an object.
4. **Movement** stand, walk, stretch; discharge the mobilised energy.
5. **Temperature and touch** cold water, a heavy blanket, pressure.
6. **Naming** "I am having a memory, not the event."

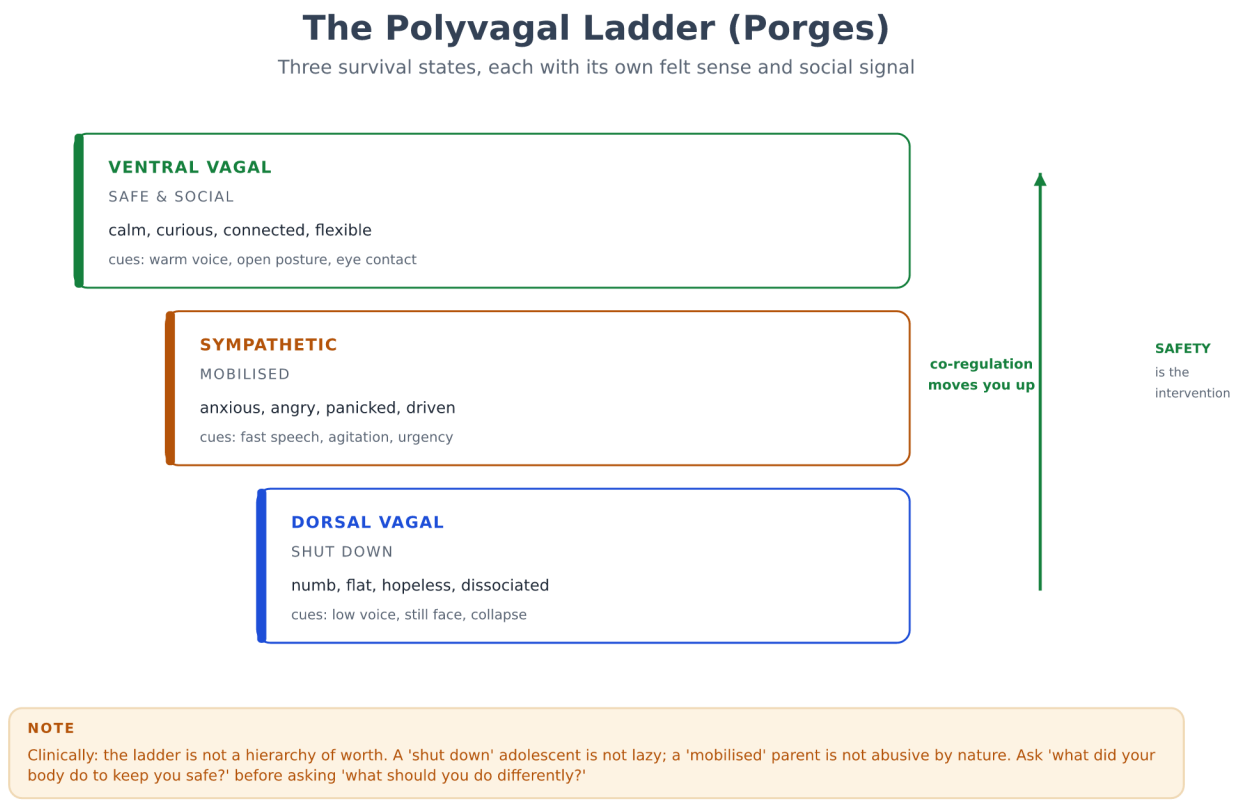


Figure 3.2 The polyvagal ladder

In family sessions. If one member leaves the window, the session has ended for them. Pause, regulate, and return. Teaching a couple to recognise each other's early signals is itself an intervention.

Week 4 Neurobiology II: memory, dissociation, the body

Traumatic memory is different. It is often implicit, sensory and fragmented: smells, images, body sensations, without a coherent narrative or a sense of "pastness". This is why "just talk about it" can flood a person and why containment matters.

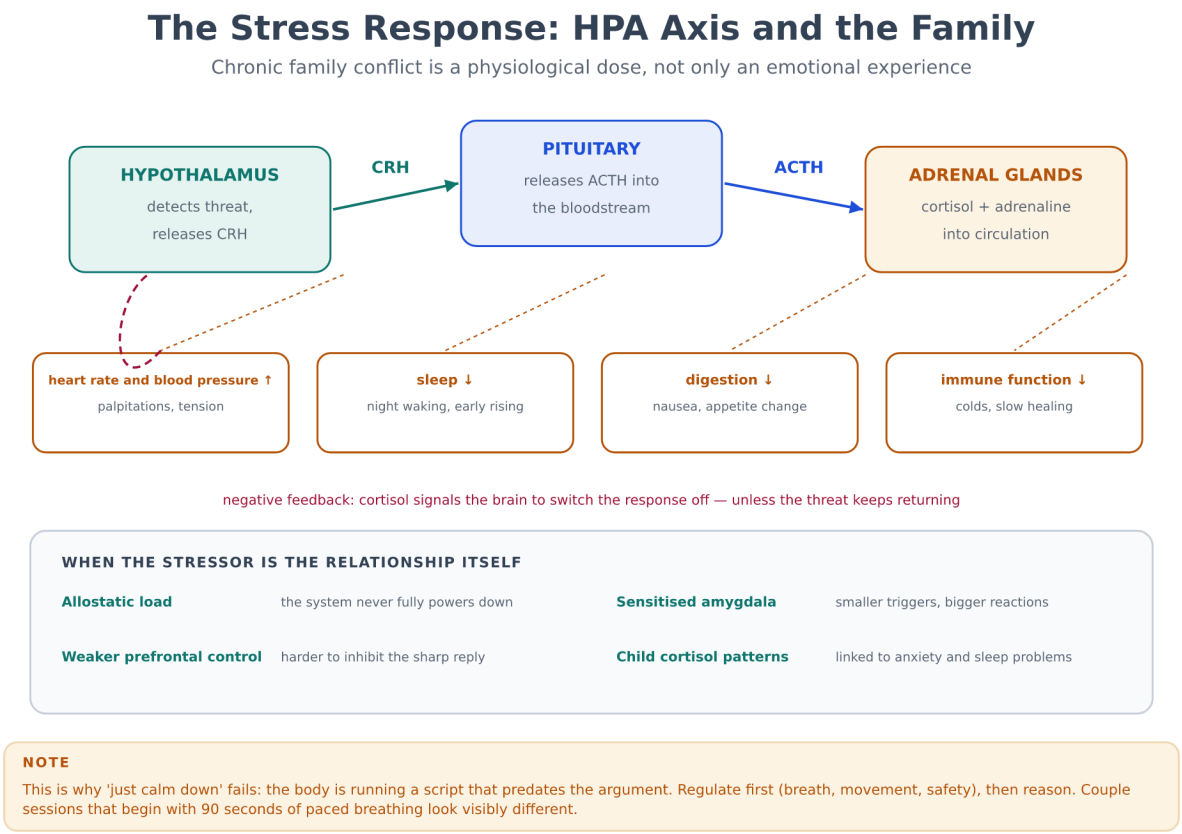


Figure 4.1 The HPA axis and chronic stress

Dissociation a disruption of consciousness, memory, identity or perception. Mild forms are common (going blank, feeling unreal); severe forms include depersonalisation, derealisation and dissociative amnesia. Assess gently: "Do you ever feel like you are watching yourself from outside? Do you lose time?"

The clinical rule. Do not interpret a dissociative response as resistance. If a client's eyes glaze, stop the exercise, orient them to the room, and slow down. Then shorten the next segment.

Week 5 Attachment and developmental trauma

Developmental trauma (sometimes "complex relational trauma") occurs when the person who should provide safety is the source of danger. The child adapts with strategies that later look like personality: vigilance, people-pleasing, aggression, emotional shutdown, compulsive caregiving.

The internal working model. "I am unlovable; others are dangerous or unreliable; I must manage alone (or never let go)." These beliefs are not irrational; they were accurate in the original environment.

Clinical implications.

1. Expect the therapeutic relationship to be tested idealisation, devaluation, testing, sudden withdrawal.
2. Predictability and reliability matter more than technique.
3. Repair after rupture is the treatment, not a distraction from it.
4. Do not interpret the transference early; be the relationship first.

Family application. Parents with their own unresolved trauma often misread their children's ordinary behaviour as threat. Psychoeducation about trauma and parenting can reduce harsh discipline dramatically.

Week 6 Trauma-informed assessment

The four R's (SAMHSA).

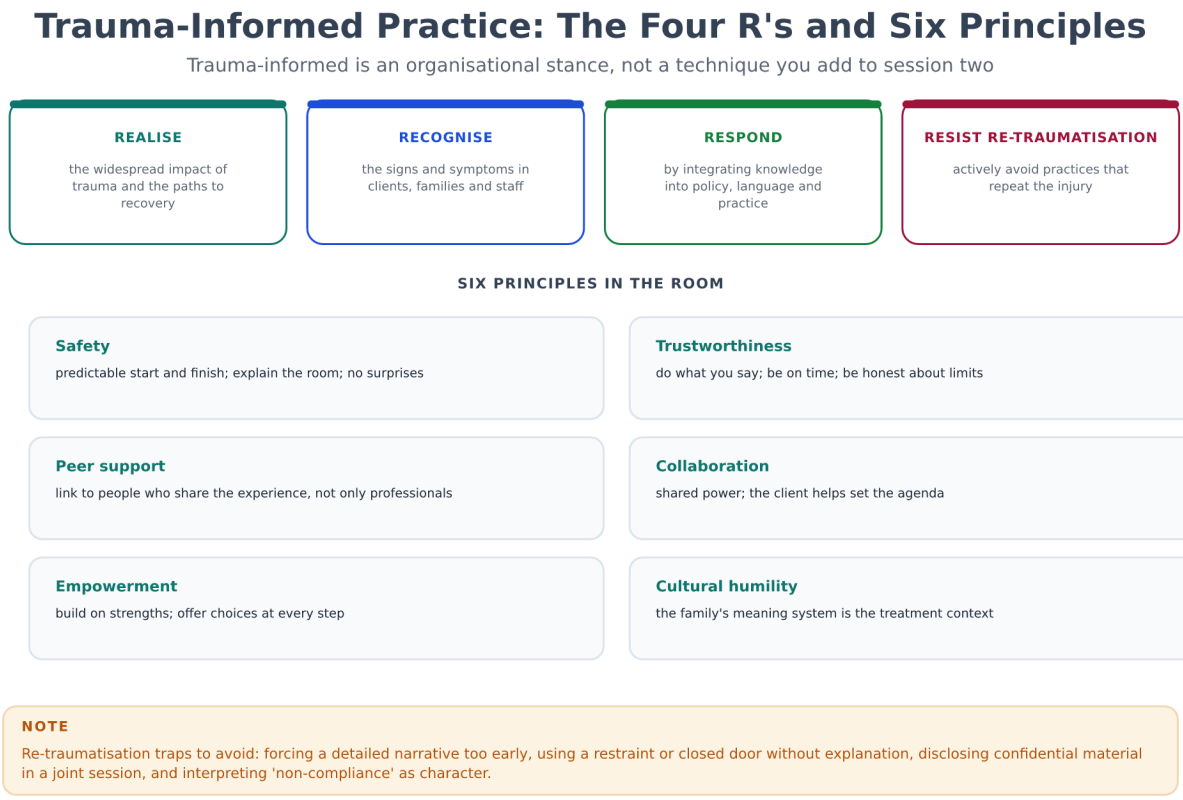


Figure 6.1 The four R's and six principles of trauma-informed practice

1. **Realise** the widespread impact of trauma.
2. **Recognise** the signs and symptoms.
3. **Respond** by integrating knowledge into practice and policy.
4. **Resist re-traumatisation.**

Assessment principles.

- Explain why you are asking, and offer choices ("you can skip any question").

- Ask about the event only at the level of detail needed for formulation; detail belongs in processing.
- Ask about **strengths, survival strategies and meaning**, not only symptoms.
- Screen for dissociation before any trauma-focused work.
- Screen for current safety (violence, self-harm, substance use).

Instruments. PCL-5 (PTSD symptoms), IES-R, CTQ (childhood trauma; use with care), DES-II (dissociation), PHQ-9, AUDIT. Always follow up a positive screen with a conversation.

Week 7 Observed trauma-informed assessment

Forty minutes. Assessed on: safety and choice throughout, pacing, grounding as needed, formulation without premature interpretation, and accurate risk screening.

Week 8 The phased model

Phase 1: Safety and stabilisation. Establish safety, build the alliance, teach regulation, address practical dangers (housing, violence, substance use), and develop what Herman called "the capacity to contain".

Phase 2: Processing. Work through the traumatic material using a validated method (TF-CBT, EMDR, narrative exposure, prolonged exposure) only when the person can tolerate it and has stabilisation skills.

Phase 3: Integration and reconnection. Rebuild identity, relationships, work, and meaning. This phase is often the longest and the most neglected.

Why sequence matters. Processing without stabilisation produces flooding, drop-out, and sometimes retraumatisation. Stabilisation without processing can produce a stable but constrained life. The judgment is clinical, not clerical: some people need two sessions of stabilisation; some need twenty.

Family application. The family's safety and stability is Phase 1 at the system level. You cannot do trauma processing in a household where the perpetrator is still present.

Week 9 Trauma-focused CBT with children

TF-CBT (Cohen, Mannarino & Deblinger) is the best-evidenced treatment for childhood trauma. Its PRACTICE components:

Component	Content
Psychoeducation and parenting skills	Trauma responses; consistent, non-punitive discipline
Relaxation	Breathing, muscle relaxation, grounding
Affect expression and modulation	Feelings identification, coping

Component	Content
Cognitive processing	Correcting trauma-related beliefs ("it was my fault")
Trauma narrative	Gradual, structured account in the child's medium (book, drawing)
In vivo mastery	Facing avoided, safe situations
Conjoint sessions	Child shares the narrative with the caregiver
Enhancing safety	Safety skills and planning for the future

The caregiver's role is essential. The non-offending caregiver's belief, protection and support is the strongest predictor of recovery. Work with them in parallel.

Week 10 Other trauma-specific modalities

Modality	Mechanism	Evidence	Cautions
EMDR	Bilateral stimulation during memory recall	Strong for PTSD	Requires training; not a magic technique
Narrative exposure therapy	Chronological testimony of the whole life	Strong for multiple/collective trauma	Can be destabilising without stabilisation
Prolonged exposure	Repeated imaginal and in vivo exposure	Strong	Drop-out; requires strong alliance
Somatic approaches	Body-based regulation and discharge	Emerging evidence	Beware of unfounded claims
Yoga / mindful movement	Regulation, interoception	Good adjunctive evidence	Not a substitute for trauma processing

The professional duty. Use what you are trained and supervised to use. A half-learned trauma protocol is more dangerous than a well-delivered stabilisation phase.

Week 11 Couples and trauma

Three presentations.

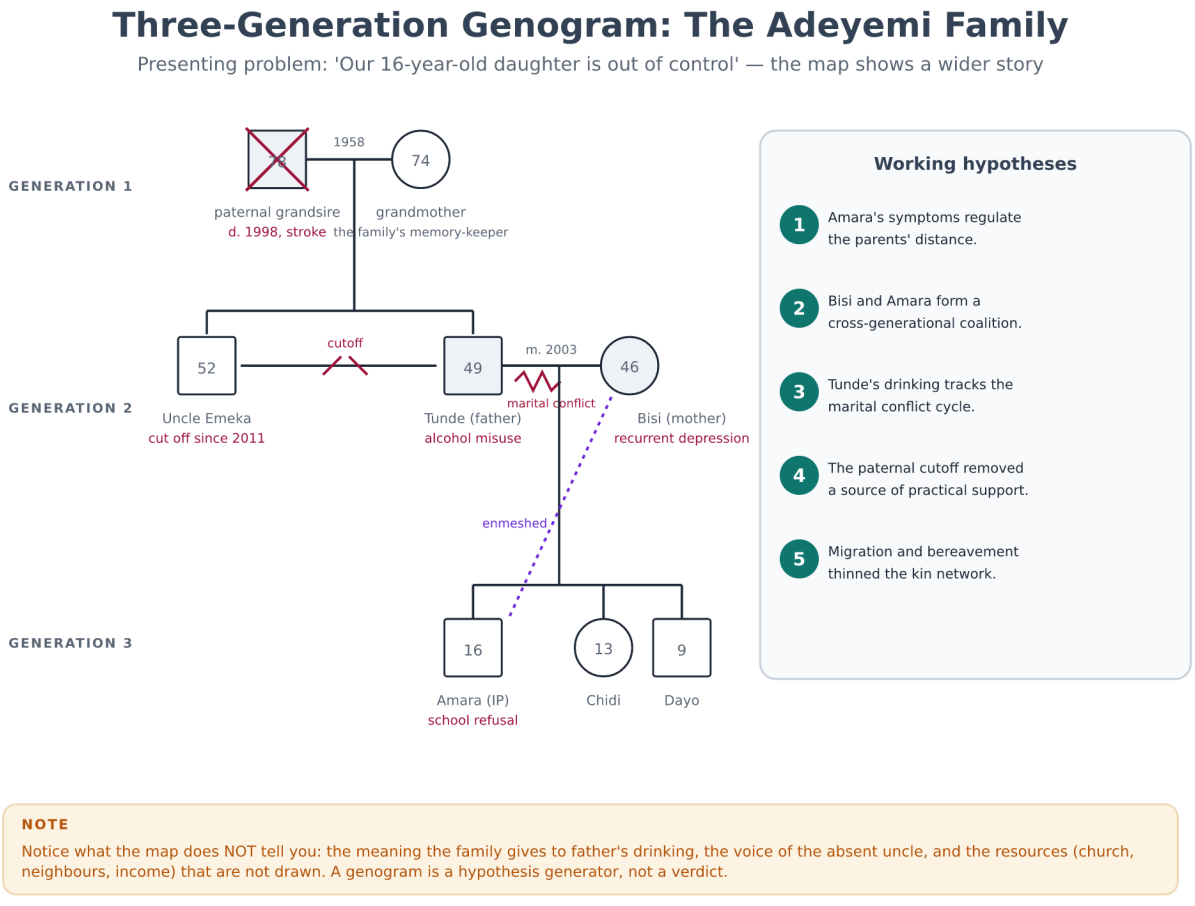
1. **One partner has a trauma history.** The other becomes a de facto carer; intimacy becomes a trigger; the couple's sexual relationship collapses. Work with both: psychoeducation for the partner, boundaries for the survivor, and a paced intimacy plan.
2. **The trauma happened inside the relationship** (betrayal, violence, coercion). This is betrayal trauma. Safety and accountability come first; joint work only where the offending partner acknowledges the harm and the victim chooses it.
3. **Both partners are trauma survivors.** Two dysregulated nervous systems in one room. The pace must be slower, the structure tighter, and the regulation explicit.

The couple-level formulation. "When she freezes, he experiences rejection and pursues; when he pursues, she freezes harder. The loop is a collision of two survival strategies, not a failure of love."

Week 12 Intergenerational transmission

Mechanisms. Parenting behaviour (frightening, inconsistent, harsh), attachment patterns, family narrative, epigenetic and stress-physiology effects, and structural factors (poverty, displacement, community violence).

The genogram of trauma. Draw three generations and mark losses, violence, separation, migration, untimely deaths and secrets. Then ask: what was never spoken about? What did each generation have to survive?



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 12.1 A three-generation genogram as a map of transmission

The hopeful finding. Transmission is probabilistic, not deterministic. A consistent, protective relationship at any point breaks the chain. Your therapy may be that relationship.

Week 13 Re-traumatisation in services

How services re-traumatise, usually without meaning to.

Practice	Why it harms	Alternative
Restraint without explanation	Replicates powerlessness	Explain, offer choices, debrief
Closed doors, no warning	Replicates entrapment	Predictability, warnings, exits
Being asked to repeat the story to every professional	Replicates helplessness	Record once, share with consent
Long waits in public areas	Replicates exposure	Private, quiet space
Disclosing in front of a partner	Replicates danger	Always screen individually
"Non-compliance" language	Replicates blame	"What got in the way?"

The organisational standard. Trauma-informed care is a policy and management issue: staff training, physical environment, restraint policy, complaint routes, and supervision. A trauma-informed clinician in a trauma-organised service will burn out.

Week 14 Vicarious trauma

Distinguish three concepts. Burnout (exhaustion from workload), compassion fatigue (reduced empathy), and vicarious trauma (changes to the therapist's beliefs and inner world from exposure to others' trauma).

Signs. Intrusion of clients' material, nightmares, cynicism, loss of hope, changed worldview ("the world is unsafe"), avoidance of trauma cases, over-involvement.

Protection. Supervision that addresses the work's emotional impact; caseload diversity; clear boundaries; your own therapy; sleep, exercise and relationships; and acknowledging meaning many clinicians find that sustained trauma work deepens their sense of purpose when it is properly supported.

Week 15 What trauma-informed care is not

It is not the avoidance of difficult material. It is not an excuse for never processing trauma. It is not a synonym for "be gentle". It is a disciplined sequencing of safety, processing and integration, delivered in a relationship and an organisation that do not repeat the injury.

Glossary

Adverse childhood experiences (ACEs) potentially traumatic events in childhood, measured cumulatively. **Betrayal trauma** trauma caused by someone on whom the person depends. **Complex PTSD** PTSD plus affect dysregulation, negative self-concept and relational disturbance. **Dissociation** disruption of consciousness, memory, identity or perception. **Grounding** techniques that return attention to the present and the body. **Re-traumatisation** repetition of the dynamics of the original trauma in the therapy or service. **Stabilisation** the first phase of trauma treatment, focused on safety and regulation. **Window of tolerance** the arousal range within which a person can think and connect.

Self-assessment

Multiple choice

1. Complex PTSD is distinguished by: (a) one event (b) affect dysregulation, negative self-concept, relational disturbance (c) psychosis (d) amnesia only

2. The first phase of trauma treatment is: (a) processing (b) safety and stabilisation (c) integration (d) exposure

3. Dissociation should first be: (a) interpreted (b) assessed and accommodated (c) ignored (d) challenged

4. TF-CBT's trauma narrative is: (a) forced in session one (b) gradual and structured, in the child's medium (c) written by the therapist (d) optional and never used

5. The strongest predictor of recovery in childhood trauma is: (a) the trauma's severity (b) a believing, protective caregiver (c) IQ (d) medication

6. Vicarious trauma is: (a) the same as burnout (b) changes to the therapist's beliefs and inner world from exposure (c) a diagnosis (d) avoidable by holidays

7. Re-traumatisation in services often occurs through: (a) privacy (b) predictability (c) restraint without explanation (d) choice

8. Processing trauma without stabilisation risks: (a) faster recovery (b) flooding and drop-out (c) improved alliance (d) no effect

9. Intergenerational transmission is: (a) deterministic (b) probabilistic and can be interrupted (c) genetic only (d) irrelevant

10. A trauma-informed organisation is characterised by: (a) only individual therapy (b) policy, environment, training and supervision aligned with trauma knowledge (c) shorter sessions (d) avoiding trauma discussion

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 c 8 b 9 b 10 b

Short answer

1. Distinguish complex PTSD from classic PTSD.
2. Describe three regulation skills and when you would use each.
3. Name the four R's of trauma-informed care.
4. Give the three phases of trauma treatment with the task of each.
5. List four ways services re-traumatise clients and one alternative for each.

Essay (choose one, 2,000 words)

1. "Trauma-informed care is a stance, not a technique." Discuss with reference to your own practice.
2. Design a trauma-informed family service: policies, environment, staffing and clinical pathway.
3. Analyse the challenges of trauma work with a couple where the trauma occurred inside the relationship.

Further reading

- Herman, J. L. (1992/2015). Trauma and Recovery. Basic Books.
- van der Kolk, B. (2014). The Body Keeps the Score. Viking.
- Cohen, J. A., Mannarino, A. P., & Deblinger, E. (2017). Treating Trauma and Traumatic Grief in Children and Adolescents (2nd ed.). Guilford.
- Porges, S. W. (2011). The Polyvagal Theory. Norton.
- SAMHSA. (2014). SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach.
- Felitti, V. J. et al. (1998). Relationship of childhood abuse and household dysfunction to many of the leading causes of death in adults. American Journal of Preventive Medicine, 14(4), 245 258.

Next: MFT 715 Family Systems Theory

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 15 OF 28

MFT 715

Family Systems Theory

15 teaching weeks 3 practicals 7 figures

SEMESTER TWO

1.71 units Prerequisite: MFT 702

> This is the theoretical anchor of the programme.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0

MFT 715 Family Systems Theory

MSc Marriage and Family Therapy Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 702

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 715 Family Systems Theory. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	**Family Systems Theory**	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 The systemic turn . . . 6

Week 2 General systems theory . . . 7

Week 3 Cybernetics I . . . 7

Week 4 Cybernetics II: first and second order . . . 8

Week 5 Communication theory . . . 9

Week 6 Structural theory . . . 11

Week 7 Multigenerational theory . . . 11

Week 8 Milan and meaning . . . 12

Week 9 Constructivism and social constructionism . . . 13

Week 10 Narrative and postmodern theory . . . 14

Week 11 Critical, feminist and anti-racist systems theory . . . 14

Week 12 Attachment and neuroscience in systems theory . . . 15

Week 13 Critiques and limitations . . . 15

Week 14 Complexity, networks and the contemporary field . . . 16

Week 15 Your theoretical position . . . 16

Glossary . . . 17

Self-assessment . . . 17

- Multiple choice . . . 17
- Answer key . . . 18
- Short answer . . . 18
- Essay (choose one, 2,000 words) . . . 18

Further reading . . . 18

Course leader's summary. This is the theoretical anchor of the programme. Where MFT 702 surveys schools, this course interrogates the theory beneath them: cybernetics, general systems thinking, structure, communication, constructionism and their critics. You will finish able to explain precisely what you mean when you say a family is "a system".

How to use this book

The word "system" is used loosely in clinical settings. This course tightens it. Each week takes one theoretical tradition, states its claims precisely, applies it to a case, and then asks what it cannot explain. You are expected to leave with arguments, not slogans.

1. Course description

Family systems theory is not one theory but a family of theories sharing a commitment: **the unit of analysis is the relationship, and the whole is more than the sum of its parts**. The course traces that commitment from von Bertalanffy's general systems theory and the Macy conferences on cybernetics, through the Palo Alto communication school, Minuchin's structuralism, Bowen's multigenerational theory, and the Milan group, to social constructionism, critical and feminist theory, and contemporary complexity and network perspectives.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the core propositions of general systems theory and cybernetics	Exam
LO2	Analyse a family using at least four theoretical traditions	Case analysis
LO3	Critique each tradition, including its cultural and political assumptions	Essay
LO4	Distinguish first- and second-order cybernetics and demonstrate reflexivity	Viva
LO5	Apply constructionist ideas to meaning-making in a family	Case analysis
LO6	Evaluate the evidence for systemic propositions	Exam
LO7	Construct a coherent theoretical position of your own	Position paper

3. Assessment

Component	Weight	Due
Theoretical analysis of a case using four traditions (2,500 words)	35%	Week 9
Essay: critique of a systemic concept (2,000 words)	25%	Week 13
Examination	25%	Week 16

Component	Weight	Due
Position paper and viva (1,000 words)	15%	Week 15

4. The fifteen-week plan

Week	Topic	Practical
1	The systemic turn: what it was and what it answered	Compare two accounts of one case
2	General systems theory: wholes, boundaries, equifinality	Systems vocabulary drill
3	Cybernetics I: feedback, control, homeostasis	Feedback mapping
4	Cybernetics II: first and second order; reflexivity	Reflexivity exercise
5	Communication theory: axioms, punctuation, paradox	Transcript analysis
6	Structural theory: organisation, hierarchy, boundaries	Structural mapping
7	Multigenerational theory: anxiety, triangles, differentiation	Triangle mapping
8	Milan and meaning: beliefs, premises, circularity	Circular questioning
9	**Theoretical analysis due**; constructivism and social constructionism	Meaning-making interview
10	Postmodern and narrative theory: knowledge and power	Texts and stories exercise
11	Critical, feminist and anti-racist systems theory	Power audit of a model
12	Attachment and neuroscience meet systems theory	Integration diagram
13	**Essay due**; critiques and limitations of systems thinking	Debate
14	Complexity, networks and the contemporary field	Network map of a family
15	Your theoretical position	Viva

Week 1 The systemic turn

What changed. Before the systemic turn, the family was understood as the environment of an individual patient. After it, the family became the unit of treatment, and individual symptoms became functions of relationship patterns.

What it answered. Three problems the individual model struggled with: Why does a symptom appear in a particular child? Why does the family resist change (homeostasis)? Why does the same individual behave differently in different relationships?

What it risked. Dissolving the person into the system; treating "the family" as a homogeneous actor; ignoring power, gender, class and history; and producing descriptions that could not be falsified.

PRACTICAL

Take one case. Write it twice: once as an individual formulation (as a CBT or psychodynamic clinician would) and once as a systemic one. Then write a paragraph on what each version makes visible and invisible.

Week 2 General systems theory

Von Bertalanffy's propositions, translated for clinicians:

Proposition	Clinical meaning
A system is a set of elements in interaction	The family is defined by its interactions, not its members
The whole is more than the sum of its parts	The family has properties no member has alone
Systems are open or closed to their environment	A family that refuses outside contact becomes closed and rigid
Systems have boundaries	Who is inside, who is outside, and how permeable
Systems are hierarchical	Subsystems nested within larger systems
Equifinality	Different starting points can reach the same end state
Equipotentiality	Similar starting points can reach different end states
Negative entropy	Systems import energy to counter disorder (information, support)

Equifinality and equipotentiality are the most useful and most neglected. Equifinality explains why two very different families produce the same symptom; equipotentiality explains why two similar families produce very different outcomes. Together they demolish simple causal stories.

Example. Two children are referred for school refusal: one in a family with high conflict and one in a family with high achievement pressure. Equifinality explains the identical presentation; equipotentiality explains why one family recovers in four sessions and the other takes two years.

Week 3 Cybernetics I

Feedback. A system regulates itself by monitoring its own output and adjusting its input.

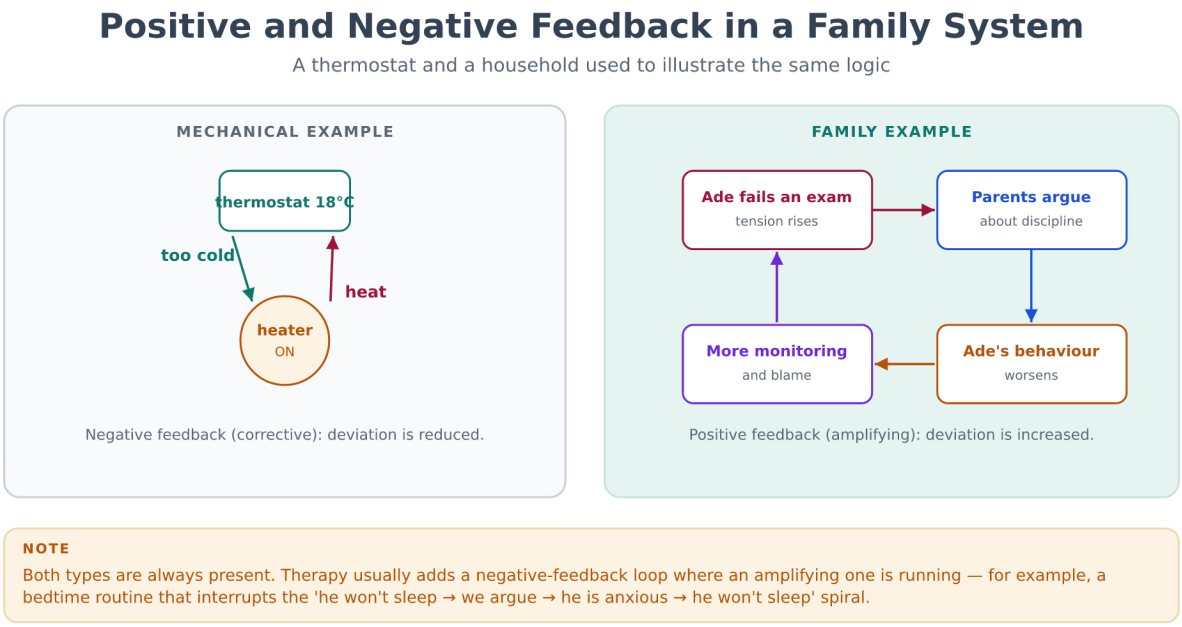


Figure 3.1 Negative and positive feedback in a machine and in a family

- **Negative feedback** corrects deviation: the thermostat, the bedtime routine, the family that de-escalates when voices rise.
- **Positive feedback** amplifies deviation: the microphone screech, the escalation, the panic spiral.

Homeostasis. The system's tendency to maintain its organisation. **Morphostasis** maintains; **morphogenesis** changes. A family can be stably unhappy: the symptom is one of the mechanisms that holds the system together.

The clinical insight. Attempts to change a system are themselves inputs. If the family's equilibrium depends on the symptom, your intervention will produce a counter-move. Predicting the counter-move and sometimes prescribing it is the strategic tradition's contribution.

Calibration example. A family's daughter has "tantrums" that end a 45-minute argument between her parents. The tantrum is negative feedback for the marital system (it restores calm) and positive feedback for the child (it works). Two feedback systems, one event.

> **PRACTICAL**
> Map both feedback loops in a disguised case: one for each "layer" (the couple system and the child's behaviour). Show how the same event serves both.

Week 4 Cybernetics II: first and second order

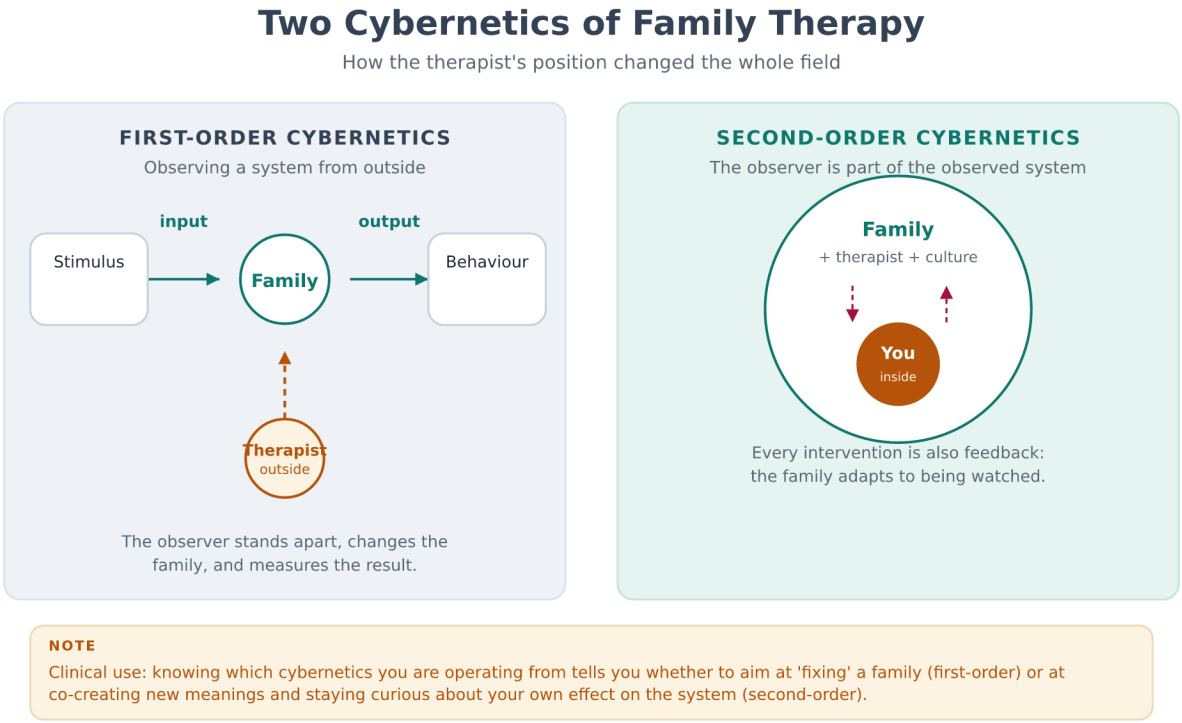


Figure 4.1 First- and second-order cybernetics: where the therapist stands

First-order cybernetics. The observer is outside the system; interventions are inputs; the aim is control of the system's output. This produced the heroic era: directive, strategic, clever.

Second-order cybernetics. The observer is part of the observed system; distinctions are drawn by an observer; knowledge is provisional. This produced curiosity, the reflecting team, and the therapist's accountability for their own effect.

The clinical consequences.

First order	Second order
"What intervention will fix this family?"	"What am I noticing, and what am I blind to?"
The family is the object of change	The family and the therapist co-construct change
Neutrality is possible	Position is unavoidable; transparency is required
Expertise is the therapist's	Expertise is distributed

Reflexivity is not self-indulgence. It is the discipline of accounting for your own contribution to the clinical picture: which family member you find easier to like, which story you believed first, which data you stopped collecting once you had a hypothesis.

Week 5 Communication theory



Figure 5.1 Watzlawick's five axioms of communication

The five axioms (Watzlawick, Beavin & Jackson).

1. One cannot not communicate.
2. Every message has a content and a relationship level.
3. Communication is punctuated into sequences by participants.
4. Messages are digital (words) and analogical (analogue).
5. Relationships are symmetrical or complementary.

Double bind hypothesis (Bateson): a person receives contradictory messages at different levels, cannot comment on the contradiction, and cannot leave the relationship. The hypothesis was weakened as an explanation of schizophrenia, but its clinical descendant recognising contradictory, un-commentable communications in families remains useful.

Critique. Communication theory could become mechanistic (families as machines exchanging messages) and could pathologise families of people with psychosis. It also tended to describe "systems" without the power differences that determine who gets to define what is communicated.

> **PRACTICAL**
> Transcript analysis: take ten lines of a couple's argument and identify punctuation, relationship-level messages, and symmetrical/complementary moves.

Week 6 Structural theory

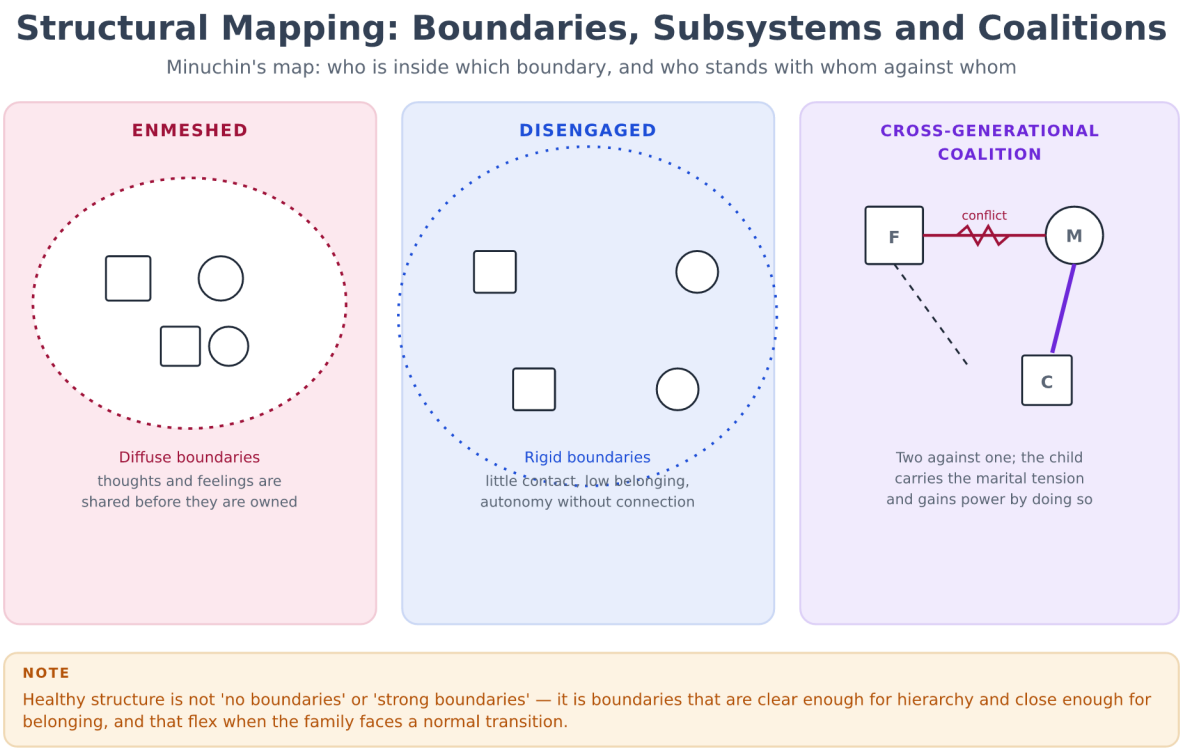


Figure 6.1 Structural mapping: enmeshed, disengaged and cross-generational coalition

Core propositions (Minuchin).

1. The family's structure is the invisible set of rules that govern transactions.
2. The structure organises the family into subsystems (spousal, parental, sibling) separated by boundaries.
3. Symptoms are best understood as the family's response to its structure, and sometimes as the only way the structure can remain stable.
4. Change in structure produces change in experience.

The evaluation criteria for a structure. Boundaries must be clear (not diffuse, not rigid); hierarchy must be appropriate to stage and culture; and subsystems must have enough autonomy to function and enough connection to belong.

Power and structure. Structural theory was criticised for assuming a normative hierarchy (executive parents, subordinate children) and for not asking how that hierarchy is upheld. Contemporary structural practice inserts the questions: whose authority is legitimate in this family, and who bears the cost of it?

Week 7 Multigenerational theory

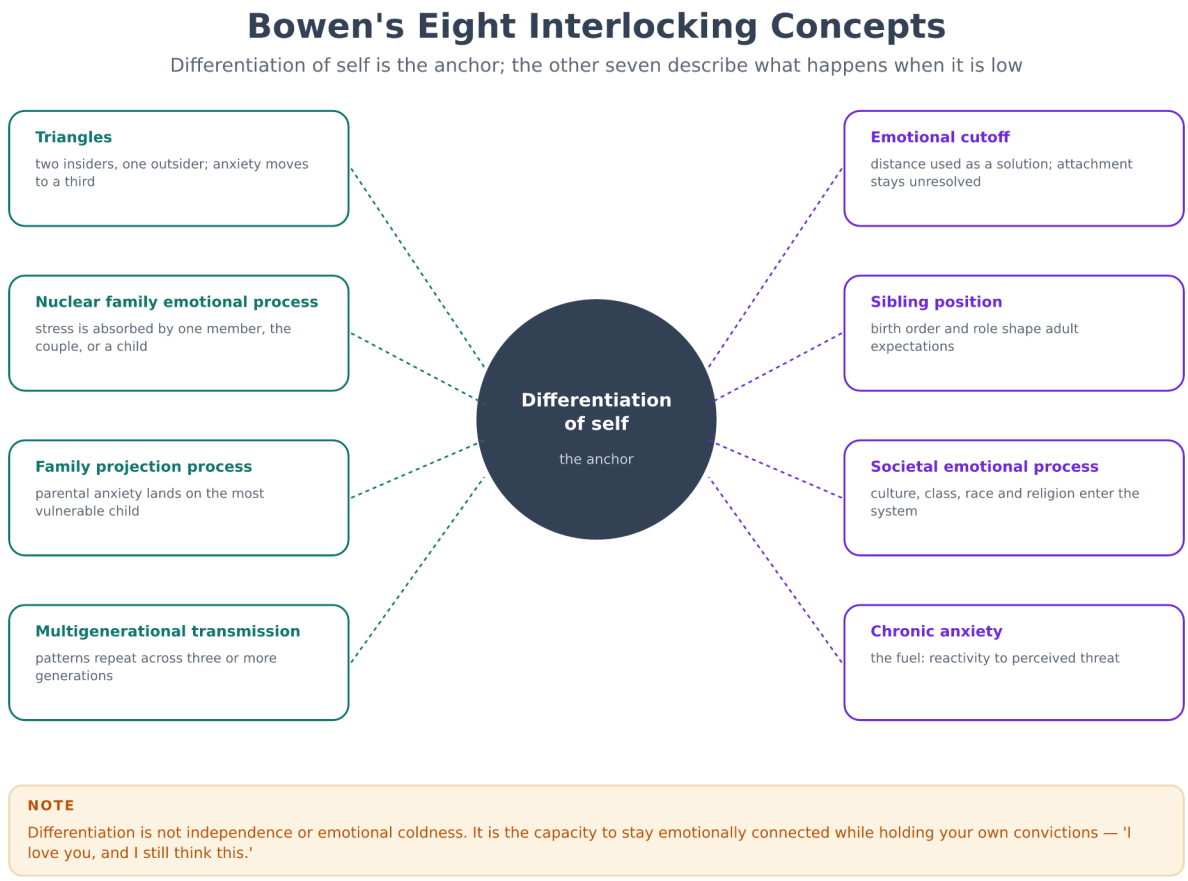


Figure 7.1 Bowen's eight interlocking concepts

Core propositions (Bowen).

1. The family is an emotional unit; anxiety is its most important variable.
2. Differentiation of self is the capacity to remain connected while thinking independently.
3. Anxiety is managed in one of four ways: conflict, distance, projection to a child, or dysfunction in one member.
4. Triangles distribute anxiety and stabilise dyads.
5. Patterns are transmitted across generations; symptomatic children are often the most triangulated.

The theory's strength is time: it offers an account of why now, by locating the present pattern in several generations of adaptation to war, migration, loss and religion.

The theory's weakness is its insufficient account of power, material constraint and culture. A woman cannot always "differentiate" out of an arranged marriage; a family cannot always "stay out of triangles" when migration has severed the network.

Week 8 Milan and meaning

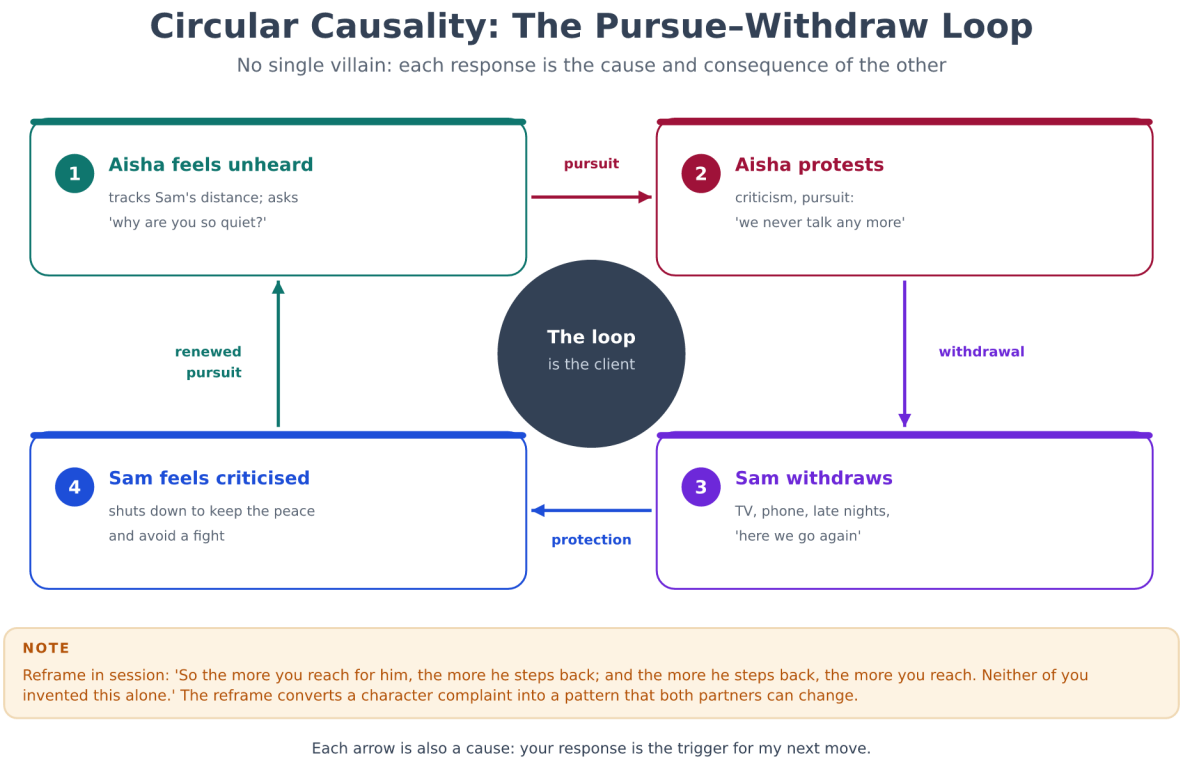


Figure 8.1 Circular causality in the consulting room

Core propositions (Palazzoli, Boscolo, Cecchin, Prata).

1. The family is a system of meanings (premises) that organise behaviour.
2. The therapist forms hypotheses and tests them with circular questions.
3. Neutrality is a stance of curiosity towards all members, not indifference.
4. Positive connotation frames all behaviour as serving the system's coherence.
5. Interventions disturb the system by changing premises, not by demanding behaviour change.

The evolution. Classical Milan used teams, rituals and directives; later Milan (and the reflecting team) abandoned directive intervention and emphasised multiple perspectives and the therapist's own openness.

Critique. Neutrality became indefensible in cases of violence and abuse; circular questioning can become an elegant evasion; and the intellectual register excluded many families.

Week 9 Constructivism and social constructionism

Constructivism (Maturana, von Glasersfeld): each person constructs a model of reality from their own experience; we cannot access an objective reality directly. Clinically: take each family member's account seriously, and stop trying to establish "the truth".

Social constructionism (Gergen, Shotter): meanings are produced between people, in language and in culture, and change with context. Clinically: the problem, the roles and the identities are made in talk, and so they can be made differently.

The distinction matters. Constructivism risks relativism and individualism (everything is "just your view"); social constructionism locates meaning in relationships and power, which makes it compatible with the critical and cultural traditions.

Implication for practice. The therapist's formulation is one description among several. Offering it humbly "here is a story I am beginning to see; how does it look to you?" is both a constructionist technique and a mark of respect.

Week 10 Narrative and postmodern theory

Narrative (White & Epston). Stories organise experience; dominant stories make alternatives invisible; the task is to externalise the problem and thicken a preferred story.

Postmodern influences. Foucault on discourse and power-knowledge; Derrida on deconstruction. The clinical translation: question the "official" account, ask who benefits from it, and look for what it silences.

The reflexivity of narrative therapy is unusual: the therapist is "decentred but influential", and the client is the author. This is an ethical stance as much as a technique.

Critique. The linguistic emphasis can under-serve families with limited literacy or in acute crisis; the evidence base, while growing, is smaller than the cognitive-behavioural tradition's.

Week 11 Critical, feminist and anti-racist systems theory

The feminist critique (Hare-Mustin 1978; Goldner; Walters) made four claims that changed the field:

1. "Circular" descriptions conceal power asymmetries.
2. "Neutrality" in the face of violence colludes with the perpetrator.
3. The gendered division of labour, not "the family system", is often the object of the wife's complaint.
4. Models that prize individuation encode a male norm.

Anti-racist and postcolonial theory extend the critique: whose family structures are treated as healthy? Which knowledge counts as evidence? How do services reproduce the state's relationship to marginalised families?

Clinical translation.

- Ask about power explicitly, in every assessment.

- Never use systemic language to make a victim responsible for the violence she receives.
- Place race, migration, class and religion inside the system, not outside it.

Week 12 Attachment and neuroscience in systems theory

Two imports that have reshaped the field.

1. **Attachment theory** gives systemic thinking a developmental mechanism: the couple and parent child systems are organised by attachment strategies, and those strategies are formed in relationships.
2. **Neuroscience** gives systemic thinking a physiology: co-regulation, the window of tolerance, mirroring and stress physiology explain how relationships change bodies.

The integration. "Systems" are not abstract patterns; they are embodied patterns between nervous systems. A couple's pursue withdraw cycle is simultaneously a communication pattern, an attachment strategy, and a sequence of autonomic states.

Critique of the imports. Neuroscience is often oversold in clinical texts; "mirror neurons" and "the right brain" are frequently invoked without evidence. Use physiology where it is well established (stress, regulation, sleep, trauma) and keep the metaphors labelled as metaphors.

Week 13 Critiques and limitations

Critique	Substance	Response
The system has no agent	"The family did it" hides who actually did what	Locate action; ask who decided
Power is under-theorised	Systemic descriptions can neutralise abuse	Explicit power analysis
Culture is externalised	"The system" was a Western two-parent family	Cultural and critical turns
Evidence is thin	Many systems concepts are unfalsifiable	Operationalise and test
The therapist disappears	Neutral observer stance	Second-order reflexivity
Complexity obscures action	Endless circular description delays intervention	Use the simplest map that guides action

The mature position. Systems theory is a disciplinary sensibility, not a complete theory. It tells you where to look (between people, across generations, in context) and what to resist (single villains and simple causes). It does not, by itself, tell you what to do.

Week 14 Complexity, networks and the contemporary field

Complexity theory. Families are non-linear, self-organising systems: small differences can produce large divergence; cause and effect are not proportional; and feedback loops are the rule. This fits clinical experience better than linear models, but it resists prediction.

Network thinking. Contemporary research treats families as networks of relationships, with density, centrality and clustering measurable. Who is central? Who is isolated? Who is the only link between two clusters? This is a formalised genogram, and it is improving our understanding of family structure and resilience.

Ecological models. Bronfenbrenner's nested systems (Figure 7 in MFT 701) place the family inside institutions, culture and time a corrective to the field's tendency to treat the family as self-contained.

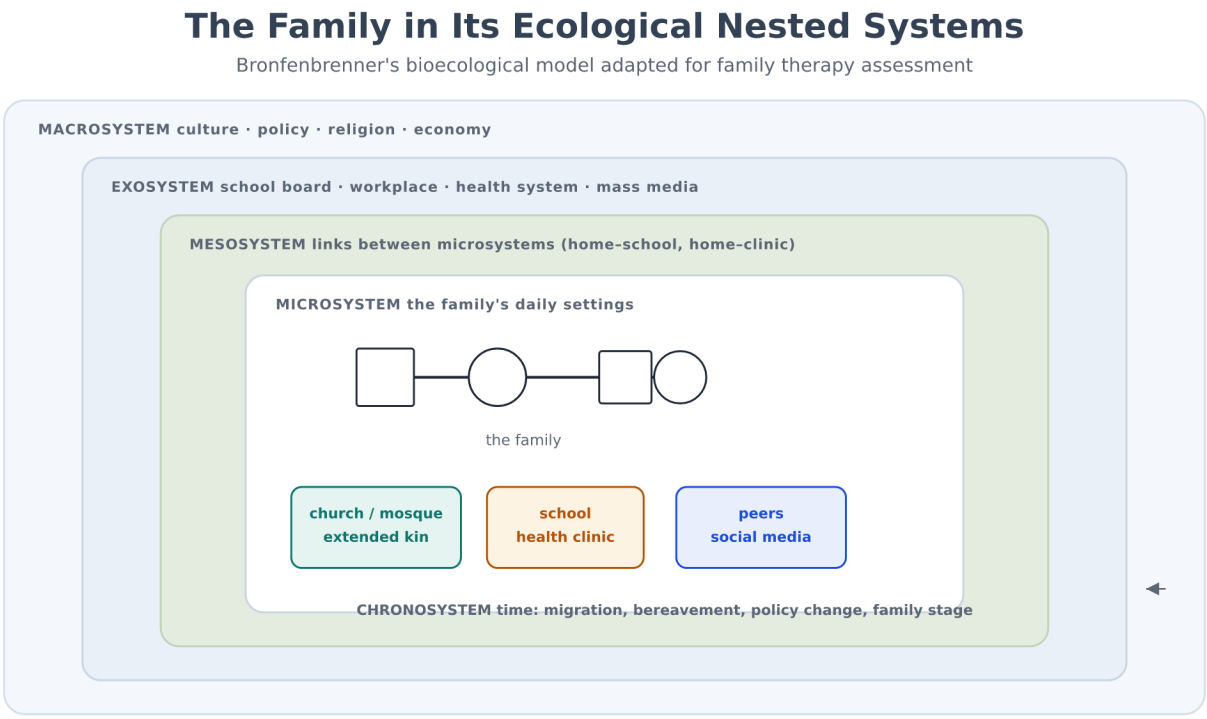


Figure 14.1 The family inside nested ecological systems

Week 15 Your theoretical position

Write 1,000 words and defend it in ten minutes:

1. What is the unit of analysis you will work with, and why?
2. What do you believe about causation in families?

3. What is the therapist's position, and how will you manage your own power?
4. Which systemic concept do you find least defensible, and what do you use in its place?
5. What would make you abandon your position?

Glossary

Autopoiesis the self-producing organisation of living systems. **Boundary** the rules governing participation and exchange between systems. **Circularity** reciprocal causation within a system. **Cybernetics** the study of control and communication in systems. **Differentiation of self** see Bowen. **Equifinality** different paths leading to the same end state. **Equipotentiality** similar starting points leading to different end states. **Homeostasis / morphostasis** the maintenance of system organisation. **Morphogenesis** change in system organisation. **Punctuation** the marking of a sequence's beginning; the source of blame. **Reflexivity** the therapist's accountability for their own participation. **Second-order cybernetics** cybernetics of observing systems. **Structure** the repetitive patterns of transaction that organise a family.

Self-assessment

Multiple choice

1. Equifinality means: (a) same cause, same outcome (b) different paths, same outcome (c) same paths, different outcomes (d) no causation

2. Homeostasis in a family refers to: (a) change (b) the maintenance of existing organisation (c) hierarchy (d) communication

3. Second-order cybernetics holds that the observer: (a) is outside the system (b) is part of the observed system (c) is irrelevant (d) is objective

4. "One cannot not communicate" is: (a) axiom 1 (b) axiom 3 (c) axiom 5 (d) a double bind

5. Punctuation is clinically important because it: (a) describes memory (b) assigns blame for a sequence (c) measures cohesion (d) is a diagnostic criterion

6. The feminist critique of family therapy argued that: (a) systems are too complex (b) circular descriptions conceal power asymmetries (c) hierarchy is unnecessary (d) therapy should be individual

7. Positive connotation is associated with: (a) structural theory (b) Milan (c) Bowen (d) behavioural therapy

8. A closed system in family terms is one that: (a) has clear boundaries (b) has limited exchange with its environment and becomes rigid (c) is disengaged (d) is enmeshed

9. Social constructionism locates meaning in: (a) the individual brain (b) relationships, language and culture (c) genetics (d) behaviour

10. The most defensible use of systems theory is as: (a) a complete predictive model (b) a disciplinary sensibility that shapes where you look (c) a diagnosis (d) a legal framework

Answer key

1 b 2 b 3 b 4 a 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Define equifinality and equipotentiality and give one clinical example of each.
2. Distinguish negative and positive feedback with a family example.
3. What are the three criteria for evaluating a family's structure?
4. State the feminist critique of "circular" formulations in four sentences.
5. What is reflexivity, and why is it a clinical skill?

Essay (choose one, 2,000 words)

1. "Systems theory tells you where to look but not what to do." Discuss.
2. Critically evaluate the concept of homeostasis in families where violence is present.
3. Compare Bowen's and Minuchin's accounts of how problems develop, and assess which is better supported.

Further reading

- von Bertalanffy, L. (1968). General System Theory. Braziller.
- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Minuchin, S. (1974). Families and Family Therapy. Harvard University Press.
- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
- Gergen, K. J. (2009). An Invitation to Social Construction (2nd ed.). Sage.
- Hare-Mustin, R. T. (1978). A feminist approach to family therapy. Family Process, 17(2), 181 194.

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 16 OF 28

MFT 716

Communication Skills in Therapy

15 teaching weeks 3 practicals 3 figures

SEMESTER THREE

1.71 units Prerequisite: MFT 706

> Every therapeutic effect passes through communication.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 716 Communication Skills in Therapy

MSc Marriage and Family Therapy Semester Three 1.71 Units 15 teaching weeks Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 716 Communication Skills in Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	**Communication Skills in Therapy**	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 Communication as the medium . . . 6

Week 2 Attending and non-verbal communication . . . 7

Week 3 Listening . . . 8

Week 4 Empathy . . . 8

Week 5 Questioning and prompting . . . 9

Week 6 Content, relationship, process . . . 9

Week 7 Feedback . . . 10

Week 8 Observed micro-skills session . . . 10

Week 9 Conflict communication and repair . . . 11

Week 10 Protocols . . . 11

Week 11 Protocol teaching test . . . 12

Week 12 Difficult conversations . . . 12

Week 13 Adaptation . . . 12

Week 14 Difficult conversation assessment . . . 13

Week 15 Written and interprofessional communication . . . 13

Glossary . . . 13

Self-assessment . . . 13

- Multiple choice . . . 13
- Answer key . . . 14
- Short answer . . . 14
- Essay (choose one, 1,800 words) . . . 14

Further reading . . . 14

Course leader's summary. Every therapeutic effect passes through communication. This course refines the therapist's own instrument listening, responding, questioning, silence, feedback, and the management of difficult conversations and teaches the protocols you will hand to families.

How to use this book

Skills are built by repetition under observation. Each week has a **micro-skill** to practise, a **protocol** to teach to families, and a **transcript** to analyse. Record yourself weekly; the gap between what you think you said and what you said is the curriculum.

1. Course description

The course covers therapeutic communication at three levels: the **micro** (attending, reflecting, summarising, questioning), the **meso** (structuring difficult conversations, giving feedback, managing conflict), and the **macro** (written communication, interprofessional communication, and communication across language and culture).

It also teaches the **structured communication protocols** used with couples and families speaker listener, the Imago dialogue, negotiation formats, family meetings and requires you to deliver them with fidelity.

2. Learning outcomes

#	Outcome	Assessed
LO1	Demonstrate micro-skills with fluency and intention	Observed session
LO2	Analyse communication at content, relationship and process levels	Transcript analysis
LO3	Teach two structured communication protocols with fidelity	Skills test
LO4	Conduct a difficult conversation (bad news, disclosure, confrontation)	Simulation
LO5	Adapt communication for children, interpreters and low-literacy contexts	Viva
LO6	Write clinical communication that is accurate and usable	Report portfolio
LO7	Identify and modify your own characteristic communication patterns	Reflective portfolio

3. Assessment

Component	Weight	Due
Transcript analysis (1,500 words)	20%	Week 6
Observed micro-skills session (30 min)	25%	Week 8
Protocol teaching test (speaker listener or Imago)	25%	Week 11

Component	Weight	Due
Difficult conversation simulation (20 min)	30%	Week 14

4. The fifteen-week plan

Week	Topic	Practical
1	Communication as the medium of therapy	Self-recording baseline
2	Attending and non-verbal communication	Non-verbal drill
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Empathy: affective, cognitive, and its limits	Empathy accuracy exercise
5	Questioning and prompting	Question taxonomy drill
6	**Transcript analysis due**; content, relationship, process	Three-level analysis
7	Feedback: giving it, receiving it, teaching families to give it	Feedback lab
8	**Observed micro-skills session**	Assessed
9	Conflict communication and repair	Repair rehearsal
10	Speaker listener and Imago dialogue	Protocol fidelity test prep
11	**Protocol teaching test**	Assessed
12	Difficult conversations: bad news, disclosure, confrontation	Simulation
13	Communication with children, interpreters and across literacy	Adaptation lab
14	**Difficult conversation assessment**	Assessed
15	Written and interprofessional communication	Report portfolio

Week 1 Communication as the medium

Three levels of every message.

1. **Content** what is said.
2. **Relationship** what it implies about the relationship ("you are the kind of person who ").
3. **Process** what is happening between the people while it is said.



Figure 1.1 Five axioms of communication

The therapist's communication is an intervention. Every reflection selects something from the client's account and returns it amplified. There is no neutral way to summarise.

Baseline recording. Record ten minutes of your own session (consented) or a role-play. Count: questions, reflections, advice statements, interruptions, and seconds of silence you initiated. Most beginners are surprised by the advice count.

Week 2 Attending and non-verbal communication

The SOLER posture (Egan): **S**it squarely, **O**pen posture, **L**ean slightly forward, **E**ye contact (culturally calibrated), **R**elax.

What families read. Pace, volume, facial expression, seating, who you look at, how you respond to distress, and whether you check your phone. A therapist who says "take your time" while glancing at the clock has said something else.

Cultural calibration. Eye contact signals respect in some cultures and challenge in others; physical distance varies; silence may indicate deference rather than resistance. Ask, or observe what the family does with each other.

Working with more than two people. Your attention is a resource that structures the family. Distribute it deliberately; narrate your choices ("I am going to come back to you, I have not forgotten"). Do not let the most articulate person become the narrator of the family.

> **DRILL**
> Triads. The therapist must convey three different messages using only non-verbal behaviour (welcome, "I need to interrupt", "take your time"). The client guesses each.

Week 3 Listening

Paraphrase (content), **reflect** (feeling), **summarise** (pattern across people and time).

Reflection levels.

Level	Example	Effect
Simple	"You are angry about the money."	Shows attention; may stall
Complex	"You are angry about the money, and underneath that, frightened that you are the only one thinking about the future."	Adds meaning; deepens
Double-sided	"You want him to be more involved, and you also do not trust him to do it right."	Holds ambivalence

The 2:1 rule. Two reflections for every question, until it becomes automatic.

What blocks listening. Formulating your next question; evaluating; wanting to reassure; your own emotional reaction; diagnostic thinking that closes the account prematurely.

Week 4 Empathy

Three components.

1. **Affective empathy** feeling something in response.
2. **Cognitive empathy** understanding the other's perspective.
3. **Empathic communication** conveying that understanding so the other feels understood.

The therapist needs all three, and needs to keep the first from driving. Affective empathy without boundaries produces over-involvement; cognitive empathy without warmth produces technique; communication without either produces scripts.

Empathy is not agreement. "I understand why you would want to leave" is empathy. "You are right to want to leave" is agreement. The first opens exploration; the second closes it and creates a coalition.

Empathic accuracy is testable. Watch a recorded session and pause before each therapist response. Write what you think the client felt. Then check with the client (or with the recording's subsequent cues).

> **DRILL**
> In pairs, one person describes a minor grievance. The listener must respond with an empathic statement that contains no advice, no question and no story of their own. Three rounds each.

Week 5 Questioning and prompting

Six families of question (from MFT 706), refined for communication work:

- **Open** "What was that like?"
- **Circular** "Who noticed first?"
- **Clarifying** "When you say 'disrespectful', what did he actually do?"
- **Reflexive** "If your daughter were listening, what would she say you both do?"
- **Scaling** "How close are you, 0 10?"
- **Confronting (gently)** "You say it does not matter, and you have mentioned it four times."

The prompt instead of the question. "Tell me more about that" often produces more than any question. Use prompts when the client is in flow; questions when you need to change direction.

Week 6 Content, relationship, process

Transcript analysis exercise. Take fifty lines of a session transcript and mark:

- every **content** statement (topic, fact)
- every **relationship-level** message ("you always ", "you are the kind of ")
- every **process** moment (something audible happening between speakers: interruption, softening, joke, silence)

Then ask: what did the therapist respond to? Beginners respond to content and miss process. Advanced practitioners respond to process and use content as evidence.

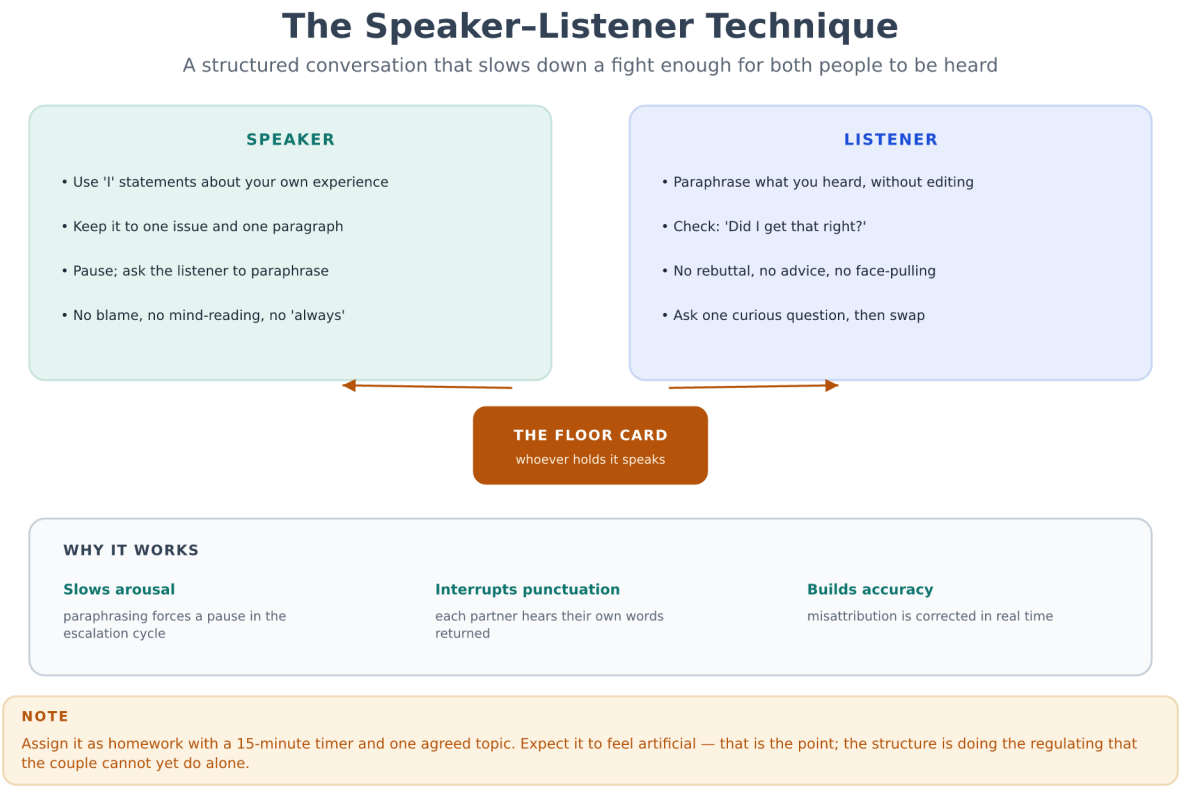


Figure 6.1 The speaker listener protocol

Week 7 Feedback

Feedback to clients should be behavioural, specific, timely and owned: "I noticed your voice dropped when you said that" (not "you are passive-aggressive").

Feedback between family members is often the missing skill. Teach the four-part structure:

1. "When you ___" (behaviour, no adjectives)
2. "I feel ___" (feeling)
3. "Because ___" (meaning)
4. "What I would like is ___" (specific request)

Feedback to colleagues and supervisees: use the same discipline. "What I saw / what it did to me / what to keep / what to try" never the sandwich, which trains people to hear praise as a warning.

Week 8 Observed micro-skills session

Thirty minutes. Assessed on: attending; reflection ratio; complex reflections; question variety; one deliberate silence; one summary; and a clean close.

Week 9 Conflict communication and repair

The escalation sequence: complaint criticism contempt defensiveness stonewalling.

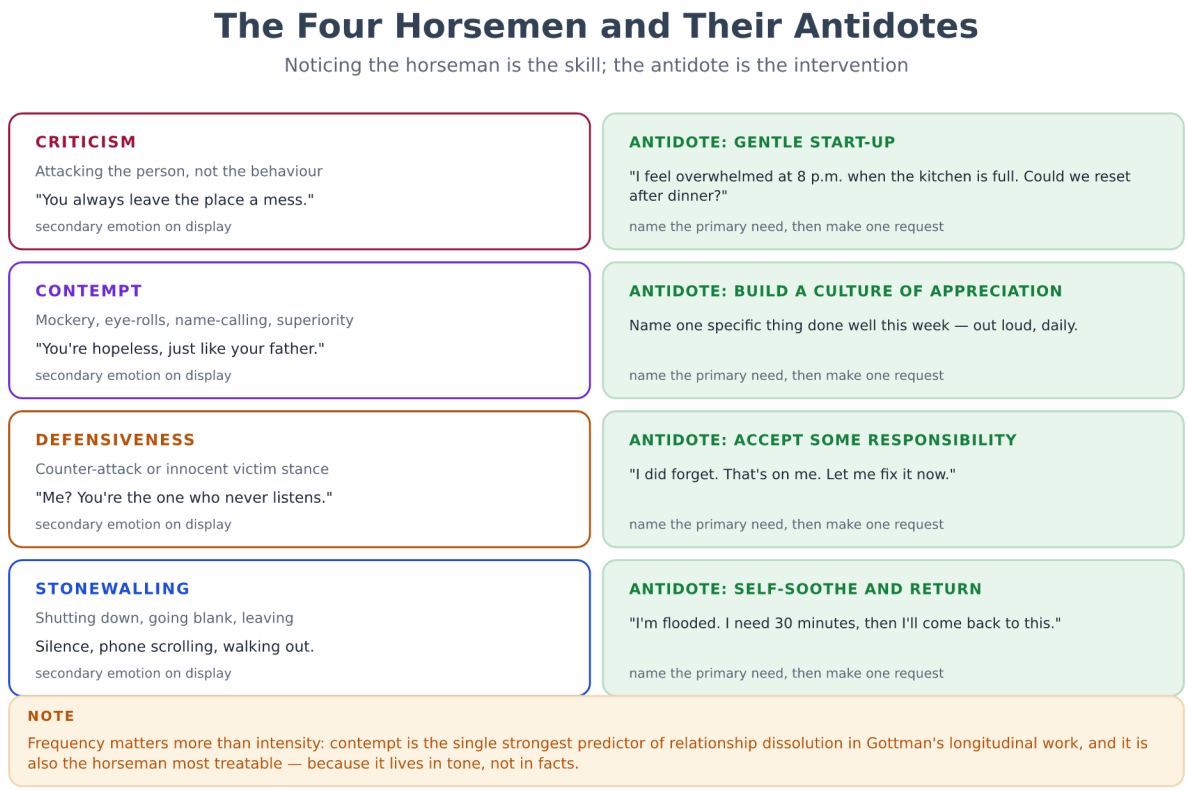


Figure 9.1 The four horsemen and their antidotes

Repair is the skill that matters. Teach couples to use a repair phrase: "Can we start again?", "I am flooded, give me twenty minutes", "That came out wrong."

The therapist's repair. You will make mistakes: mis-summarise, miss a disclosure, take a side. Repair in the moment: "I got that wrong. Let me try again." Modelling repair is more powerful than describing it.

Week 10 Protocols

Speaker listener. A floor card, a timer, paraphrase before responding, then swap. Teach it, rehearse it in session, and set it as homework with one agreed topic.

Imago dialogue. Three steps: mirror ("so what I heard you say is "), validate ("that makes sense because "), empathise ("I imagine you feel "). More demanding than speaker listener, and more powerful with practised couples.

Negotiation format. Define one issue; each states a position; generate three options; choose one to trial for two weeks; review.

Fidelity matters. A protocol delivered loosely is a protocol that fails, and the couple will conclude that "communication exercises do not work". Practise until you can teach it in ten minutes without notes.

> **ASSESSED**
> Teach speaker listener to a role-played couple in twelve minutes, then coach them through one round. Assessed on clarity, demonstration, coaching, and management of a partner who will not follow the rules.

Week 11 Protocol teaching test

As above, assessed formally, with feedback.

Week 12 Difficult conversations

Four types and their structures.

1. **Bad news** (a diagnosis, a death, a safeguarding outcome). Structure: warning shot ("I have some difficult news") information in small chunks check understanding silence emotional response plan follow-up. Do not fill the silence after the news.
2. **Disclosure** (a secret that must be shared, an affair, a risk). Structure: prepare the person agree the content be present manage the response plan.
3. **Confrontation** (a boundary violation, a safety concern, a colleague's conduct). Structure: describe the behaviour its impact the expectation the consequence the support available.
4. **Ending therapy when it is not working** (referral, rupture, incompetence). Structure: name the difficulty own your part offer a route document.

The common failure is rushing to reassurance. "At least " and "it will be fine" terminate the conversation the client needs to have.

Week 13 Adaptation

Children. Short sentences, concrete language, check understanding by asking them to explain it back, and use a medium.

Interpreters. Speak to the family, first-person interpretation, brief and debrief (see MFT 708).

Low literacy. Avoid written-only instructions; use drawings, demonstration, and teach-back ("show me how you would do it").

Hearing or cognitive impairment. Reduce background noise, face the person, one idea per sentence, and confirm comprehension.

Second-language sessions. Slow down, avoid idiom, check meaning ("when you say 'respect', what does that include?").

Week 14 Difficult conversation assessment

Twenty-minute simulation drawn from a bank (bad news, disclosure, confrontation, ending). Assessed on: structure, pacing, emotional attunement, clarity of the message, and a documented follow-up.

Week 15 Written and interprofessional communication

Clinical notes. Behavioural, dated, factual, and signed. Avoid adjectives about people; avoid speculation.

Referral letters. One page: reason, relevant history, what you have tried, what you are asking for, and who to contact.

Reports for courts and schools. Answer the question asked; state the limits of your expertise; separate fact from opinion.

Interprofessional communication. Use SBAR (Situation, Background, Assessment, Recommendation). It is short, it survives interruption, and it is what other professions are trained to receive.

Glossary

Complex reflection a reflection that adds meaning or feeling beyond the client's words. **Double-sided reflection** a reflection that holds both sides of an ambivalence. **Empathic accuracy** the degree to which a therapist correctly infers a client's experience. **Mirroring** repeating back the other's words as the first step of a structured dialogue. **Process** the relational layer of communication, distinct from content. **Repair** an attempt to de-escalate and reconnect after a rupture. **SBAR** Situation, Background, Assessment, Recommendation: a handover format. **SOLER** a posture mnemonic for attending behaviour.

Self-assessment

Multiple choice

1. The recommended ratio of reflections to questions for beginners is: (a) 1:1 (b) 2:1 (c) 1:2 (d) 5:1

2. "You are right to want to leave him" is: (a) empathy (b) agreement (c) a reflection (d) a summary

3. A complex reflection: (a) repeats content (b) adds meaning or feeling (c) asks a question (d) gives advice

4. The main error in delivering bad news is: (a) pausing (b) rushing to reassurance (c) checking understanding (d) using plain language

5. SBAR stands for: (a) Situation, Background, Assessment, Recommendation (b) Summary, Behaviour, Action, Review (c) Safety, Boundaries, Alliance, Repair (d) Session, Block, Ask, Respond

6. In Imago dialogue the three steps are: (a) mirror, validate, empathise (b) ask, tell, repeat (c) listen, advise, conclude (d) reflect, question, summarise

7. Fidelity in teaching a communication protocol matters because: (a) it is required by codes (b) a loosely delivered protocol fails and discredits the method (c) it is faster (d) clients prefer it

8. Eye contact should be: (a) constant (b) culturally calibrated (c) avoided (d) used only with adults

9. The 2:1 rule exists to: (a) reduce interrogation and enrich material (b) fill time (c) avoid emotion (d) demonstrate technique

10. A referral letter should include: (a) the full case file (b) reason, relevant history, what you tried, the request, contact (c) only a diagnosis (d) your opinion of the family

Answer key

1 b 2 b 3 b 4 b 5 a 6 a 7 b 8 b 9 a 10 b

Short answer

1. Describe the three levels of a message with one example of each.
2. Give three examples of a complex reflection on the same client statement.
3. What are the four steps of the feedback structure for family members?
4. Outline the structure for delivering bad news.
5. Name four adaptations for a family with limited literacy.

Essay (choose one, 1,800 words)

1. Analyse a transcript of your own session: what did you respond to, and what did you miss?
2. "Repair, not avoidance, is the mark of a skilled communicator." Discuss with clinical examples.
3. How should communication protocols be adapted for a family in which the couple does not share a first language?

Further reading

- Egan, G. (2013). The Skilled Helper (10th ed.). Cengage.
- Miller, W. R., & Rollnick, S. (2013). Motivational Interviewing (3rd ed.). Guilford.

- Gottman, J. M., & Silver, N. (2015). The Seven Principles for Making Marriage Work. Harmony.
- Hendrix, H., & Hunt, H. (2004). Getting the Love You Want. Holt.
- Silverman, J., Kurtz, S., & Draper, J. (2013). Skills for Communicating with Patients (3rd ed.). Radcliffe.

Next: MFT 717 Psychopathology in Families

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 17 OF 28

MFT 717

Psychopathology in Families

15 teaching weeks · 1 practical · 1 worked case

SEMESTER THREE

1.71 units · Prerequisite: MFT 707, 715

> “Severe mental distress is not caused by families, and it is not treated without them.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0 · 2026 edition

MFT 717 — Psychopathology in Families

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 707, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 717 — Psychopathology in Families. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	**Psychopathology in Families**	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 — The historical legacy . . . 6

Week 2 — Psychosis . . . 7

Week 3 — Expressed emotion and family intervention . . . 7

Week 4 — Family psychoeducation . . . 8

Week 5 — Bipolar disorder . . . 8

Week 6 — Depression in relational context . . . 9

Week 7 — Severe anxiety, OCD and accommodation . . . 9

Week 8 — Observed psychoeducation session . . . 9

Week 9 — Personality-level difficulties . . . 10

Week 10 — Eating disorders . . . 10

Week 11 — Caregiver burden and the household . . . 10

Week 12 — Medication and collaboration . . . 11

Week 13 — Risk, hospitalisation and the family . . . 11

Week 14 — Recovery, stigma and the future . . . 12

Week 15 — Integration . . . 12

Glossary . . . 12

Self-assessment . . . 12

- Multiple choice . . . 12
- Answer key . . . 13
- Short answer . . . 13
- Essay (choose one, 2,000 words) . . . 13

Further reading . . . 13

Course leader's summary. Severe mental distress is not caused by families, and it is not treated without them. This course builds the knowledge and the nerve to work with psychosis, mood disorders, severe anxiety, personality-level difficulties and eating disorders in a relational frame — alongside medication and psychiatry, not instead of them.

How to use this book

The course deliberately reverses a historical mistake. For decades, family therapy implied that families caused severe mental illness; that legacy still harms families who arrive expecting to be blamed. Each week therefore begins with what is known about aetiology, then asks what a family therapist actually contributes.

1. Course description

The course covers the major categories of severe psychopathology as they present in family life: psychotic disorders, bipolar disorder, major depression, severe anxiety and OCD, personality-level difficulties, eating disorders, and the interface with substance use. It emphasises **family psychoeducation**, **expressed emotion**, **family accommodation**, **relapse prevention**, **caregiver burden**, the **recovery model**, and respectful collaboration with psychiatric services.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the aetiology and family impact of major disorders	Exam
LO2	Deliver family psychoeducation for psychosis or bipolar disorder	Skills test
LO3	Assess and reduce expressed emotion and family accommodation	Case study
LO4	Work with caregiver burden and family adaptation	Case study
LO5	Collaborate with psychiatric services and understand medication	Viva
LO6	Avoid both blaming and neglecting the family	Reflective portfolio
LO7	Apply a recovery-oriented frame	Position paper

3. Assessment

Component	Weight	Due
Family psychoeducation session observed (40 min)	30%	Week 8
Case study: family intervention for severe mental illness (2,500 words)	35%	Week 12
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Historical legacy: from schizophrenogenic mothers to family partnership	Timeline and critique
2	Psychosis I: what it is and what families see	Symptom and experience mapping
3	Psychosis II: family intervention and expressed emotion	EE assessment
4	Family psychoeducation in practice	Session delivery
5	Bipolar disorder and the family	Relapse signature mapping
6	Depression in relational context	Couple-based depression work
7	Severe anxiety, OCD and family accommodation	Accommodation reduction plan
8	**Psychoeducation observed**	Assessed
9	Personality-level difficulties and the family	Boundary and validation work
10	Eating disorders and the family	FBT principles
11	Caregiver burden, siblings and the household	Carer support plan
12	**Case study due**; medication, psychiatry and collaboration	Medication interface
13	Risk, hospitalisation and the family	Admission and discharge planning
14	Recovery, stigma and the family's future	Recovery plan
15	Integration: partnership without blame	Case presentations

Week 1 — The historical legacy

The mistake. Mid-twentieth-century theories blamed mothers for schizophrenia ("schizophrenogenic mother"), attributed psychosis to family communication ("double bind"), and described "families of origin" as pathogenic. These ideas were poorly evidenced, deeply damaging, and are still remembered by families.

The correction. Contemporary research shows that families are neither the cause nor the cure of psychotic disorders, but they are among the strongest predictors of outcome — through support, monitoring, accommodation and emotional climate.

What family therapy offers now:

- psychoeducation (reliable information, in plain language)
- communication and problem-solving training

- reduction of criticism, hostility and over-involvement where they are high
- relapse prevention and early-warning planning
- support for carers
- help with the ordinary relational life that the illness interrupts

> **PRACTICAL**
> Read one historical text and one contemporary family-intervention trial. Write 400 words on what changed, and what harm the earlier framing did.

Week 2 — Psychosis

What it involves. Hallucinations, delusions, disorganised thinking, negative symptoms (flat affect, anhedonia, withdrawal), and cognitive impairment. Onset is typically late adolescence or early adulthood, which means it lands in the middle of a family's launching stage.

What families experience. Fear, grief, confusion, exhaustion, stigma, violence in a minority of cases, and a chronic state of vigilance. Siblings often report invisibility; parents often report marital strain.

What helps (evidence summary). Antipsychotic medication is the foundation. Family intervention (typically 10–12 sessions of psychoeducation plus communication and problem-solving) reduces relapse and improves functioning. Supported employment, cognitive remediation and long-term follow-up matter.

What to avoid. Blaming; interpreting delusions symbolically; colluding with the illness ("he cannot help it, so nothing is required of him"); and treating the family as the patient.

Week 3 — Expressed emotion and family intervention

Expressed emotion (EE) comprises criticism, hostility and emotional over-involvement. High EE predicts relapse in schizophrenia and other conditions, though the effect is smaller than early research suggested and is bidirectional (illness severity also raises EE).

Assessing EE without blaming. Ask about the last month: how many arguments, what triggered them, how the carer responded, how much time is spent managing symptoms, and whether the carer feels able to have a life of their own.

The compassionate reframe. High criticism is usually exhaustion; over-involvement is usually love and fear. Naming that — before teaching any skill — is what makes the intervention acceptable.

Script: "You have been managing something terrifying for four years. Of course you are exhausted and sometimes sharp. The aim of these sessions is not to make you nicer; it is to give you back some room."

Communication training then teaches: making positive requests, active listening, expressing negative feelings safely, and problem-solving in structured steps.

Week 4 — Family psychoeducation

The format. 6–12 sessions, ideally including the person with the diagnosis and their carers, with a combination of information, communication training and problem-solving.

A single-session structure.

Segment	Content
1. What the family already knows	Elicit their model of the illness
2. What is known	Symptoms, course, causes (multi-factorial), treatment, prognosis
3. What helps	Medication adherence, sleep, stress reduction, early warning signs, routines
4. What the family can do	Communication, problem-solving, reducing accommodation, protecting their own life
5. What to do in a crisis	Numbers, triggers, what to say, who to call
6. Questions	Ten minutes of honest answers

Language. Plain, non-technical, and hopeful without being false: "This is a serious condition that can be managed. It is not anyone's fault, and it is not a life sentence."

Week 5 — Bipolar disorder

What families see. Periods of elevated mood, reduced sleep, grandiosity, irritability, risk-taking, and then depression. The family's experience is often of a person they recognise only intermittently.

Family tasks. Recognising the **relapse signature** (early signs of an episode in this person), agreeing a plan in advance, protecting sleep, managing money during highs, and responding to risk.

The relapse signature plan.

1. List the person's own early signs (e.g. sleeping 4 hours and feeling fine; starting three projects).
2. Agree what the family will do (e.g. call the psychiatrist; take over the bank card; reduce stimulation).
3. Agree what the family will not do (argue about whether an episode is real; wait for a crisis).
4. Review after each episode.

Psychoeducation plus family-focused treatment has good evidence for relapse prevention in bipolar disorder.

Week 6 — Depression in relational context

Two directions. Depression is influenced by relationships (criticism, conflict, isolation, caregiving burden), and it influences them (withdrawal, irritability, reduced libido, helplessness).

Couple-based interventions have good evidence for depression, particularly where relationship distress is present. Key mechanisms: reducing criticism and hostility, increasing positive exchange, addressing attributional blame, and supporting behavioural activation with a partner's help.

What not to do. Treat the depressed person as the sole problem; ask the partner to become a therapist; ignore the couple's sexual and practical reality; or reassure prematurely.

> **CASE**
> A husband's depression follows a job loss. The couple's cycle: he withdraws, she takes over the household, he feels useless, she feels abandoned. Intervention: behavioural activation with agreed tasks that restore his role, plus work on her anxiety about relinquishing control.

Week 7 — Severe anxiety, OCD and accommodation

Family accommodation is the family's participation in the anxiety: answering the same question forty times, checking locks, avoiding supermarkets, providing reassurance, taking over tasks.

Why it persists. It reduces distress immediately, which reinforces it for both parties.

Reducing accommodation without cruelty. Build a hierarchy with the family; reduce one item at a time; agree what the person will do instead; expect a temporary increase in distress; and rehearse the script: "I love you, and I am not going to answer that again. I will sit with you for ten minutes instead."

For OCD specifically: psychoeducation about the function of compulsions; family agreement not to participate in rituals; ERP as the treatment of choice, with the family as allies.

Week 8 — Observed psychoeducation session

Forty minutes: eliciting the family's model, delivering information in plain language, teaching one communication skill, agreeing a crisis plan, and answering questions honestly.

Week 9 — Personality-level difficulties

Caution with language. "Personality disorder" is a contested and stigmatising label. Many clinicians describe "personality-level difficulties" or "complex relational patterns" and focus on the pattern rather than the category.

What families experience. Emotional storms, self-harm, idealisation and devaluation, fear of abandonment, and often years of contact with services that have not helped.

What helps.

1. **Validation before change:** "Given what you have survived, this reaction makes sense."
2. **Consistency:** the same rules, the same times, the same responses.
3. **Boundary clarity:** "I will call you back at six" — and doing it.
4. **Family psychoeducation** about emotion regulation and invalidation.
5. **Coordination between services**, because splitting between professionals is part of the pattern.

What harms. Moralising; interpreting the behaviour as manipulation; inconsistency; and treating every crisis as a new emergency rather than part of a known pattern.

Week 10 — Eating disorders

The relational reality. Eating disorders reorganise the household: meals become battlegrounds, siblings become collateral, and parents oscillate between control and helplessness.

Family-based treatment (FBT) for adolescent anorexia is first-line. The stance is **agnostic about cause** and **externalising of the illness**: the family defeats anorexia, and no one is blamed for it.

Phase 1: parents take charge of refeeding, with support, and are absolved of guilt. **Phase 2:** control is returned to the adolescent as developmentally appropriate. **Phase 3:** identity and adolescent issues.

Medical safety is primary. Eating disorders have the highest mortality of any mental disorder. Weight, electrolytes and cardiac status are medical matters; do not work alone.

For adults with longstanding eating disorders, the work is often individual and relational: shame, identity, control, and the couple's accommodation.

Week 11 — Caregiver burden and the household

Assess the carers, not only the patient. Burden is measurable and treatable: use a brief carer measure and ask directly about sleep, work, finances, social life, and their own health.

Siblings. Often invisible, often parentified, often resentful and guilty about the resentment. Give them a session, language for their experience, and permission to have their own life.

The household system. Who is doing the caring? Who has given up what? Is there a "designated carer" whose own life has been suspended? Naming this is often the most useful intervention of the month.

Carer support plan. Respite, information, peer support groups, practical help, and their own GP.

Week 12 — Medication and collaboration

What you need to know (not prescribe).

Class	Common uses	Common side effects to ask about
Antipsychotics	Psychosis, bipolar	Weight gain, sedation, tremor, sexual dysfunction
Antidepressants (SSRIs)	Depression, anxiety, OCD	Nausea, sexual dysfunction, initial agitation
Mood stabilisers (lithium, valproate)	Bipolar	Thirst, tremor, weight, thyroid and renal monitoring
Benzodiazepines	Short-term anxiety	Dependence, sedation, falls

Your role. Support adherence without policing it; explore ambivalence about medication as you would any other ambivalence; report side effects; never advise stopping or changing a dose; and know the prescriber's name.

Collaboration. Write a short letter; attend a case conference; use SBAR on the phone; agree who is responsible for what. Families are often caught between two professionals who have never spoken.

Week 13 — Risk, hospitalisation and the family

Admission is a family event: relief, guilt, fear, and often secrecy. Prepare the family, explain the process, agree visiting, and plan from the first day for discharge.

Discharge is the highest-risk period. Plan: medication supply, follow-up appointment, who is at home, what the early warning signs are, and what to do at night.

When the family is afraid. Take the fear seriously; it is usually based on experience. Build a written crisis plan that the whole household knows.

Week 14 — Recovery, stigma and the future

The recovery model emphasises hope, identity, meaning and self-determination, not only symptom reduction. For families, this means asking: what does a good life look like for this person, and what would support it?

Stigma operates inside the family too: shame, secrecy, "we do not tell anyone", blame of the mother. Addressing internalised stigma is a clinical task.

Planning the future. Housing, work, relationships, and what happens when the parents can no longer care. This conversation is difficult and usually deferred; the therapist's job is to make it possible.

Week 15 — Integration

Four principles.

1. **No blame.** Families are partners, not pathogens.
2. **Information is an intervention.** Accurate, plain-language psychoeducation changes outcomes.
3. **Emotion is contagious.** Reducing criticism and over-involvement helps — and it is achieved by supporting carers, not by instructing them.
4. **Collaboration beats isolation.** Medication, psychiatry, community services and family work together.

Glossary

Accommodation — family behaviours that participate in and maintain symptoms. **Expressed emotion (EE)** — criticism, hostility and emotional over-involvement in carers. **Family-focused treatment (FFT)** — a manualised programme of psychoeducation, communication and problem-solving, with good evidence in bipolar disorder. **Family psychoeducation** — structured information and skills for families living with a disorder. **Prodrome** — the early signs preceding a full episode. **Recovery model** — an orientation emphasising hope, identity, meaning and self-determination. **Relapse signature** — an individual's characteristic early warning signs of an episode.

Self-assessment

Multiple choice

1. Contemporary research on families and psychosis shows that families: (a) cause schizophrenia (b) can significantly influence outcome (c) are irrelevant (d) should be excluded

2. High expressed emotion comprises: (a) criticism, hostility, over-involvement (b) warmth, humour (c) avoidance (d) grief

3. Family accommodation in OCD: (a) reduces long-term symptoms (b) maintains symptoms despite short-term relief (c) is irrelevant (d) should be punished

4. FBT for adolescent anorexia is: (a) blame-based (b) agnostic about cause and externalises the illness (c) individual therapy (d) medication-only

5. A relapse signature is: (a) a diagnosis (b) an individual's early warning signs (c) a blood test (d) a medication

6. The highest-risk period after psychiatric admission is: (a) admission (b) discharge (c) the first year (d) none

7. The therapist's role regarding medication is to: (a) prescribe (b) adjust doses (c) support adherence and report side effects (d) advise stopping

8. Siblings of a person with severe mental illness commonly report: (a) no impact (b) invisibility and guilt about resentment (c) greater support (d) reduced burden

9. The recovery model emphasises: (a) symptom removal only (b) hope, identity, meaning, self-determination (c) institutional care (d) family blame

10. The most damaging historical legacy for families was: (a) too much medication (b) theories blaming families for causing psychosis (c) short admissions (d) family therapy

Answer key

1 b · 2 a · 3 b · 4 b · 5 b · 6 b · 7 c · 8 b · 9 b · 10 b

Short answer

1. State the evidence for family intervention in psychosis in four sentences.
2. What is expressed emotion, and how do you assess it compassionately?
3. Outline a single-session family psychoeducation structure.
4. Give three ways to reduce family accommodation without cruelty.
5. What should a discharge plan contain?

Essay (choose one, 2,000 words)

1. "Families are partners, not pathogens." Trace the history of this claim and its clinical implications.
2. Design a family intervention for a young person with first-episode psychosis, including the first four sessions.
3. Critically discuss the ethics of using the diagnosis "personality disorder" with a family in the room.

Further reading

- Leff, J., & Vaughn, C. (1985). Expressed Emotion in Families. Guilford.

- Kuipers, E., Leff, J., & Lam, D. (2002). Family Work for Schizophrenia (2nd ed.). Gaskell.
- Miklowitz, D. J. (2008). Bipolar Disorder: A Family-Focused Treatment Approach (2nd ed.). Guilford.
- Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa (2nd ed.). Guilford.
- National Institute for Health and Care Excellence. (2014/2020). Psychosis and Schizophrenia in Adults (CG178).

Next: MFT 718 — Intimacy and Relationship Dynamics

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 18 OF 28

MFT 718

Intimacy and Relationship Dynamics

15 teaching weeks 1 practical 1 worked case 1 figure

SEMESTER THREE

1.71 units Prerequisite: MFT 710

> Intimacy is not a single thing, and sex is not the whole of it.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 718 Intimacy and Relationship Dynamics

MSc Marriage and Family Therapy Semester Three 1.71 Units 15 teaching weeks Prerequisite: MFT 710

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 718 Intimacy and Relationship Dynamics. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	**Intimacy and Relationship Dynamics**	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 What intimacy is . . . 6

Week 2 Attachment and intimacy . . . 7

Week 3 Desire . . . 8

Week 4 The sexual response . . . 8

Week 5 Taking a sexual history . . . 9

Week 6 Talking about sex . . . 9

Week 7 Desire discrepancy . . . 10

Week 8 Erectile and arousal difficulties . . . 10

Week 9 Pain, orgasm and low desire . . . 11

Week 10 Betrayal and repair . . . 11

Week 11 Culture, religion and sexual scripts . . . 11

Week 12 Consent, coercion and obligation . . . 12

Week 13 Lifespan and special populations . . . 12

Week 14 Sensate focus . . . 13

Week 15 The therapist's comfort . . . 13

Glossary . . . 13

Self-assessment . . . 14

- Multiple choice . . . 14
- Answer key . . . 14
- Short answer . . . 14
- Essay (choose one, 2,000 words) . . . 14

Further reading . . . 15

Course leader's summary. Intimacy is not a single thing, and sex is not the whole of it. This course integrates attachment, desire, communication, culture and the body and gives you language for the conversations most couples cannot start.

How to use this book

The material is intimate by design. You will be asked to examine your own beliefs about sex, bodies, gender and obligation, because those beliefs will shape what you notice and what you avoid. Supervision is essential for this course; several exercises should be processed there.

1. Course description

The course covers the psychology and physiology of intimacy and desire, the major sexual concerns presenting in couple therapy, the relational dynamics of pursuit and distance, betrayal and repair, the effects of culture and religion, and the practical skills of talking about sex.

It is explicitly **not** a sex therapy certification. It is the level of competence a family therapist needs to assess sexual concerns confidently, intervene relationally, and refer appropriately.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess intimacy across multiple domains	Observed assessment
LO2	Explain desire, arousal and response, including discrepancy	Exam
LO3	Conduct a respectful sexual history	Skills test
LO4	Formulate sexual concerns relationally and identify medical rule-outs	Case study
LO5	Deliver sensate focus and communication interventions	Skills test
LO6	Address culture, religion and consent	Reflective portfolio
LO7	Recognise the limits of your competence and refer	Viva

3. Assessment

Component	Weight	Due
Sexual history observed (30 min)	30%	Week 6
Case formulation and intervention plan (2,500 words)	40%	Week 11
Examination	20%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What intimacy is: eight domains	Domain assessment
2	Attachment and intimacy	Attachment-informed formulation
3	Desire: responsive and spontaneous	Desire mapping
4	The sexual response and its variation	Physiology teaching practice
5	Taking a sexual history	Observed practice
6	**Sexual history assessed**; talking about sex with couples	Language lab
7	Desire discrepancy and negotiation	Negotiation rehearsal
8	Common concerns I: erectile and arousal difficulties	Biopsychosocial formulation
9	Common concerns II: pain, orgasm, low desire	Referral decision-making
10	Betrayal, infidelity and the repair of trust	Disclosure planning
11	**Case plan due**; culture, religion and sexual scripts	Cultural genogram of sex
12	Consent, coercion and obligation	Boundary and consent work
13	Sexuality across the lifespan and in special populations	Adaptation planning
14	Sensate focus and intimacy exercises	Exercise design
15	Integration: the therapist's own comfort	Viva

Week 1 What intimacy is

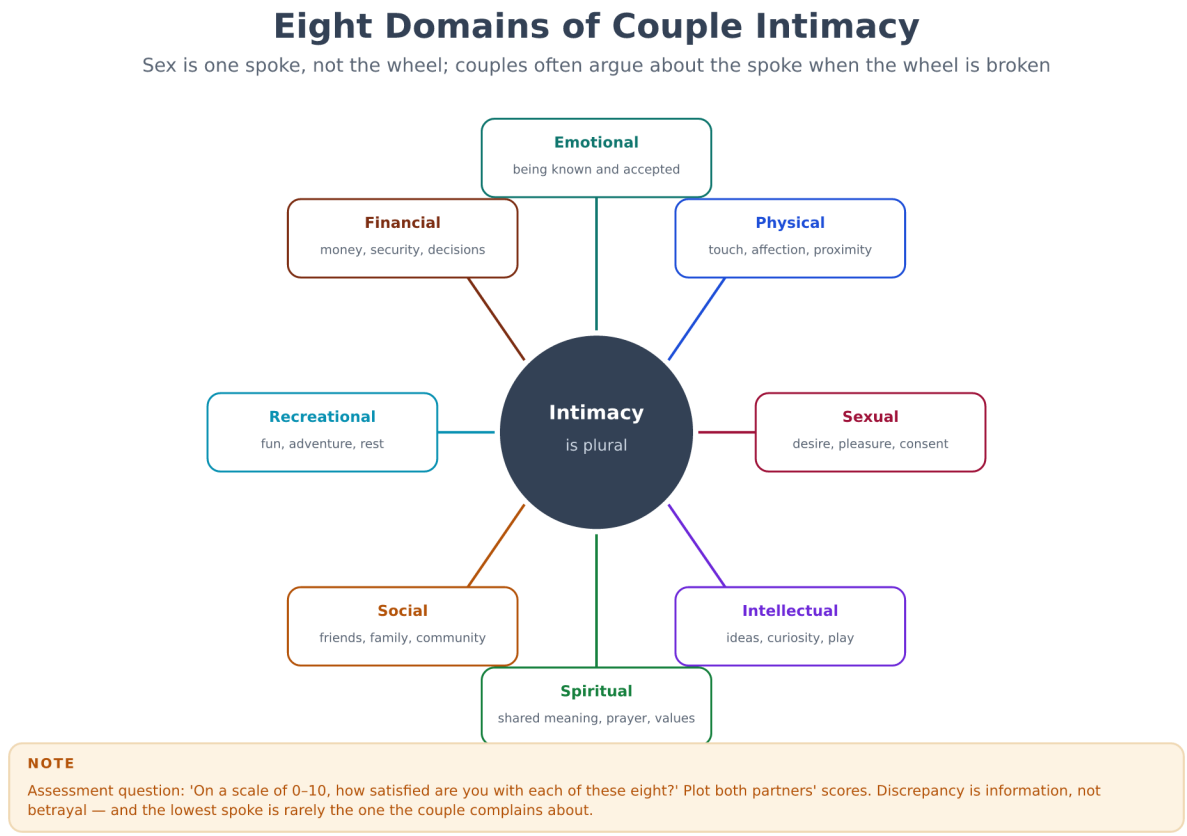


Figure 1.1 Eight domains of couple intimacy

Couples rarely complain about "intimacy". They complain about sex, or loneliness, or a partner who is always on the phone. Your task is to locate the complaint in the wider system of intimacy domains: emotional, physical, sexual, intellectual, spiritual, social, recreational, financial.

The common pattern. A couple presents with a sexual problem, and assessment reveals that the emotional domain has been starved for years. Treating the sexual symptom alone usually fails; rebuilding the other domains usually helps the sexual one.

Assessment question. "On a scale of 0 to 10, how satisfied are you with each of these eight areas?" Plot both partners' answers. The gap between them, and the gap between the couple's stated problem and their lowest domain, is the clinical map.

Week 2 Attachment and intimacy

Secure attachment allows both closeness and separateness: the person can ask for comfort and can tolerate a partner's independence.

Anxious attachment produces hypervigilance: sex may be used to secure reassurance, and any distance feels like rejection.

Avoidant attachment produces discomfort with dependency: sex may be easier than emotional closeness, and post-coital distance is common.

Disorganised attachment produces approach avoidance: desire and fear in the same moment.

Clinical application. "What does sex mean to each of you?" often reveals the attachment function: proof of love, proof of adequacy, a way to avoid conversation, a way to end a conflict, a rare moment of safety.

Week 3 Desire

Two models.

- **Spontaneous desire**: desire appears before arousal, seemingly unbidden. More commonly reported by men in many samples, and by people in new relationships.
- **Responsive desire**: desire emerges after arousal, in response to context, closeness, relaxation or stimulation. More commonly reported by women, by parents of young children, and by people under stress.

The clinical error. Treating responsive desire as "low desire" or "dysfunction". If a person's desire is responsive, the intervention is context time, privacy, relaxation, non-sexual affection not pressure to "want it more".

The dual control model (Bancroft & Janssen): sexual response depends on both excitation and inhibition. Stress, shame, performance anxiety, religious guilt and fear of pregnancy all raise inhibition. Lowering inhibition is often more effective than increasing stimulation.

Week 4 The sexual response

The classic model (Masters & Johnson; Kaplan): desire excitement plateau orgasm resolution. **The circular model** (Basson) is more accurate for many people, especially women: intimacy, closeness, context and stimulation interact repeatedly, without a fixed sequence.

What clinicians need to know.

- Arousal is not desire, and lubrication is not consent or interest.
- Arousal non-concordance is normal: the body can show arousal without desire, and desire without arousal.
- Orgasm is not the goal; pleasure and connection are.
- Pain is always a medical question first (vaginismus, vulvodynia, endometriosis, infection, scarring, menopause).
- Medication (especially SSRIs) commonly affects desire, arousal and orgasm.
- Ageing changes speed and reliability, not the capacity for pleasure.

Week 5 Taking a sexual history

Principles. Ask permission; explain why; use the person's language; be matter-of-fact; do not react with surprise, approval or disapproval; and take a history from each partner separately at least once.

A workable sequence.

1. Permission: "I ask everyone about this because it is often part of what brings couples here. Is that all right?"
2. Current: "How is intimacy between you at the moment? How often, and how satisfying, from 0 to 10?"
3. History: "Was there a time it was good? What was different then?"
4. Meaning: "What does sex mean to you closeness, relief, duty, play, reassurance?"
5. Body: "Any pain, any difficulty with arousal or orgasm, any medical condition or medication?"
6. History of harm: "Has anyone ever pressured or forced you sexually?" (Ask individually.)
7. Culture and faith: "What were you taught about sex growing up? What do you believe is allowed?"
8. Expectations: "What would you like to be different and what would your partner say you want?"

Language. Avoid euphemism and avoid clinical jargon. Use the words the couple uses, and offer a plain alternative if they have none.

> **ASSESSED**
> A 30-minute sexual history with a role-played client. Assessed on permission, pacing, breadth, non-judgemental tone, and the identification of medical or safety issues requiring referral.

Week 6 Talking about sex

Why couples cannot. Shame, fear of hurting, fear of being judged, previous rejection, cultural prohibition, and the belief that "if you loved me you would know".

The therapist's job is to make the conversation normal, structured and safe. Techniques:

- **The third object.** Talk about "the sex thing" as an object between you, rather than about the people.
- **Scaling.** "How satisfied are you, 0 10?" Numbers are less exposing than adjectives.
- **Written disclosure.** Each writes three sentences about what they want more of; you read them aloud.
- **The permission sentence.** "I am going to ask something direct. You can say pass."

Scripts to give couples.

> "I want to talk about us, and I am nervous. Can we take twenty minutes on Saturday?" "I miss being close to you. I am not asking for sex tonight; I am asking whether we can talk about it this week." "When you turn away, I tell myself you do not want me. Is that what you mean?"

Week 7 Desire discrepancy

The most common sexual complaint in couple therapy. It is a normal difference, not a disorder, until it produces distress.

The pattern. The higher-desire partner pursues; the lower-desire partner avoids; the pursuit increases pressure; the avoidance increases; both feel rejected, and the couple stops touching altogether.

Intervention.

1. **Normalise.** "Desire differences are the norm, not the exception. The problem is not the difference; it is the cycle around it."
2. **De-pressurise touch.** Reintroduce non-sexual affection with an explicit ban on sex for a period.
3. **Negotiate.** What is acceptable to both? Frequency is a poor target; quality, initiation and affection are better ones.
4. **Address the context.** Sleep, children, work, resentment, body image, medication.
5. **Reject the scoreboard.** Sex is not a debt; "I did it last time" is a symptom of the problem, not a solution.

Week 8 Erectile and arousal difficulties

Always rule out medical causes: vascular disease, diabetes, hypertension, medication, hormonal factors, alcohol and smoking, neurological conditions. Refer to a physician; do not treat as purely psychological.

The psychological pattern. Performance anxiety monitoring arousal drops confirmation of the fear increased anxiety. The partner's disappointment (real or imagined) intensifies it.

Relational intervention. Reduce performance focus; reintroduce sensate focus; involve the partner in non-demanding intimacy; address the couple's broader pattern; and refer for medical assessment and, where appropriate, medication.

Arousal difficulties in women follow a similar logic, with a stronger contribution from context, pain, relationship quality and hormonal change.

Week 9 Pain, orgasm and low desire

Presentation	First question	Relational work
Painful intercourse	Has a medical cause been excluded?	Pace, consent, fear, communication
Difficulty reaching orgasm	Is this new? Is it situational?	Reduce goal focus; explore what works alone
Absent or low desire	Is it global or partner-specific?	Context, resentment, responsive desire model
Aversion	Is there a history of coercion or trauma?	Safety, trauma-informed pacing, referral

Referral is not failure. Persistent pain, unexplained physical symptoms, severe trauma responses and psychiatric complexity belong with specialists. The relational work continues alongside.

Week 10 Betrayal and repair

Betrayal trauma (developed in MFT 710). The betrayed partner's symptoms mimic PTSD: intrusion, hypervigilance, intrusive images, and a shattered assumption that the partner is safe.

Repair requires four things, in order.

1. **Safety** the behaviour has stopped; boundaries are in place; no violence.
2. **Disclosure** enough truth to end the lying, not so much detail that it becomes a weapon. Agree what will be asked and answered.
3. **Remorse that is not self-pity** "I did this, it was my choice, and I see what it cost you."
4. **Consistency over time** transparency, reliability, and patience with the betrayed partner's cycling.

The cycling is normal. Recovery is not linear: the betrayed partner may be calm for two weeks and then devastated by a song. Warn both partners, so the cycle does not become a second betrayal.

Sex after betrayal. Often the couple either avoids sex entirely or uses it to short-circuit the conversation. Both are understandable and neither is repair. Rebuild slowly, with explicit consent and no expectation.

Week 11 Culture, religion and sexual scripts

Sexual scripts are culturally learned instructions about who should want what, when, and with whom.

Questions for the couple (each individually first).

- What were you taught about sex before marriage? By whom?
- What did you learn about your body?
- What is permitted, and what is forbidden? What happens if you break the rule?
- Is sex a duty, a gift, a right, a sin, a reward?
- Who initiates? Who is allowed to refuse? What happens when someone refuses?

Clinical themes in Nigerian and similar contexts.

- Pre-marital abstinence teaching that produces guilt after marriage.
- The expectation that sex is a marital obligation, which can silence refusal.
- Female sexual pleasure as unspoken or stigmatised.
- Bride price and the sense of "ownership" it can create.
- Family planning, fertility pressure, and the stigma of childlessness.
- Religious counselling that frames desire as sin.
- Polygyny, and the sexual dynamics between co-wives.

The clinical stance. You are not there to adjudicate the script; you are there to make it speakable, and to ensure that consent is real for both people.

> **CASE PLAN (40%)**
> A couple with a desire discrepancy and a history of religious guilt. Includes sexual history, relational formulation, medical rule-outs, intervention plan, and referral criteria.

Week 12 Consent, coercion and obligation

Consent is ongoing, specific and freely given. It can be withdrawn at any point, including during sex, and including in marriage.

Coercion includes pressure, sulking, threats, guilt, economic withholding, and the use of religion. It does not have to be physical to be harmful.

Assess individually: "Can you say no to your partner without fear of what will happen? What does happen when you say no?"

When there is coercion, the work is with the person experiencing it first: safety, validation, and options. Couple work is contraindicated until safety is established.

Week 13 Lifespan and special populations

Group	Considerations
Parents of young children	Exhaustion, touch saturation, interrupted privacy; plan rather than wait for spontaneity
Midlife	Hormonal change, body image, teenagers in the house, career strain

Group	Considerations
Later life	Assumption of asexuality by families and clinicians; health, medication, bereavement, new partners
Chronic illness and disability	Access, pain, fatigue, body image, and the partner's carer role
LGBTQ+ couples	Minority stress, disclosure, family rejection, and the absence of affirming services
Asexual people	Absence of sexual attraction is not pathology; negotiate intimacy on other terms
Post-cancer	Body change, fertility, fear, and the couple's avoidance

The clinician's assumption to avoid: that everyone wants sex, that everyone should, and that a relationship without sex is deficient. Ask what this couple wants.

Week 14 Sensate focus

Purpose. To reduce performance anxiety and rebuild non-demanding physical intimacy.

Progression.

1. **Non-genital touch, no sex.** Twenty minutes, one partner touches, the other receives; then swap. Feedback only on what felt pleasant, not on what was missing.
2. **Genital touch, still no intercourse.** Continue the same structure.
3. **Intercourse permitted but not required.** Often the couple is advised to continue the exercises first.
4. **Generalisation.** Gradually return to the couple's own sexual repertoire.

Rules. No goal, no intercourse during the first stages, no negative feedback, and a clear opt-out at any time. Couples who "fail" at sensate focus are usually still performing.

Week 15 The therapist's comfort

Your own sexual history, beliefs and discomfort will shape this work. Write 500 words on: what you find hardest to ask; what judgements you notice; and what you will do when you are out of your depth. Then defend it in a viva, and take it to supervision.

Glossary

Arousal non-concordance mismatch between physiological arousal and subjective desire. **Desire discrepancy** a difference between partners in the frequency or intensity of sexual desire. **Dual control model** sexual response as the balance of excitation and inhibition. **Responsive desire** desire that emerges in response to arousal or context. **Sensate focus** graded, non-demanding touch exercises. **Sexual script** culturally learned expectations about sexual behaviour and meaning. **Spontaneous desire** desire experienced before arousal, without an obvious trigger.

Self-assessment

Multiple choice

1. Lubrication indicates: (a) consent (b) physiological arousal, which may not match desire (c) desire (d) orgasm

2. Responsive desire is best supported by: (a) pressure to initiate (b) context, relaxation and non-sexual affection (c) performance goals (d) frequency targets

3. The most common sexual complaint in couple therapy is: (a) infidelity (b) desire discrepancy (c) pain (d) infertility

4. Sensate focus requires: (a) intercourse from stage one (b) no goal and no intercourse initially (c) feedback on what is missing (d) daily practice

5. Erectile difficulties should first prompt: (a) psychodynamic exploration (b) medical rule-out (c) couple blame (d) medication advice

6. Consent in marriage is: (a) automatic (b) ongoing, specific and revocable (c) implied by the ceremony (d) not a clinical matter

7. Betrayal trauma symptoms resemble: (a) depression only (b) PTSD (c) psychosis (d) nothing

8. Asexual identity: (a) is a disorder (b) is not pathology; intimacy may be negotiated otherwise (c) requires treatment (d) means no relationships

9. Painful intercourse is first: (a) a relational problem (b) a medical question (c) a sign of avoidance (d) untreatable

10. The therapist should refer when: (a) the couple mentions sex (b) persistent pain, trauma complexity or issues beyond competence arise (c) the couple is young (d) culture differs

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Name the eight domains of intimacy and explain why the lowest is not always the complaint.
2. Distinguish spontaneous and responsive desire and give the clinical implication.
3. Write six questions from a sexual history.
4. What are the four stages of sensate focus?
5. How do you assess for coercion without leading the client?

Essay (choose one, 2,000 words)

1. "Desire discrepancy is a relationship problem, not a disorder." Discuss with a case.

2. Analyse the influence of religious teaching on sexual intimacy in couple therapy.
3. Discuss the ethical and clinical issues in working with sex in a couple where coercion is suspected.

Further reading

- Basson, R. (2001). Human sex response cycles. Journal of Sex and Marital Therapy, 27(1), 33 43.
- Bancroft, J., & Janssen, E. (2000). The dual control model of male sexual response. Neuroscience & Biobehavioral Reviews, 24(5), 571 579.
- Leiblum, S. R. (Ed.). (2007). Principles and Practice of Sex Therapy (4th ed.). Guilford.
- Johnson, S. M. (2019). Attachment Theory in Practice. Guilford.
- Perel, E. (2006). Mating in Captivity. Harper.
- Nagoski, E. (2019). Come as You Are (rev. ed.). Simon & Schuster.

Next: MFT 719 Family Life Cycle and Transitions

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 19 OF 28

MFT 719

Family Life Cycle and Transitions

15 teaching weeks 3 practicals 2 figures

SEMESTER THREE

1.71 units Prerequisite: MFT 703, 715

> Most families come to therapy during a transition they did not choose, or one they chose but did not anticipate.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 719 Family Life Cycle and Transitions

MSc Marriage and Family Therapy Semester Three 1.71 Units 15 teaching weeks Prerequisite: MFT 703, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 719 Family Life Cycle and Transitions. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	**Family Life Cycle and Transitions**	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 Why transitions produce symptoms . . . 6

Week 2 The six stages . . . 8

Week 3 Normative and non-normative transitions . . . 8

Week 4 Horizontal and vertical stressors . . . 8

Week 5 Leaving home and joining . . . 9

Week 6 Young children . . . 10

Week 7 Adolescents . . . 10

Week 8 Launching and the empty nest . . . 11

Week 9 Later life . . . 11

Week 10 Migration and displacement . . . 11

Week 11 Observed transition plan session . . . 12

Week 12 Divorce, remarriage and blended families . . . 12

Week 13 Births, deaths and unfinished transitions . . . 12

Week 14 Rituals . . . 13

Week 15 Integration: the family in time . . . 13

Glossary . . . 13

Self-assessment . . . 14

- Multiple choice . . . 14
- Answer key . . . 14
- Short answer . . . 14
- Essay (choose one, 2,000 words) . . . 14

Further reading . . . 15

Course leader's summary. Most families come to therapy during a transition they did not choose, or one they chose but did not anticipate. This course teaches you to locate the symptom in time: at which stage is this family, which transition is it facing, and what must be renegotiated.

How to use this book

The life-cycle framework is a diagnostic instrument, not a moral timetable. Each week applies it to a different stage and a different kind of transition, including the ones that do not fit the classic sequence migration, polygyny, foster care, sudden death, disability. Culture is not an exception to the model; it is the material from which the timetable is made.

1. Course description

The course covers the Carter McGoldrick stage model; the distinction between predictable (normative) and unpredictable (non-normative) transitions; horizontal and vertical stressors; the family's need to renegotiate roles, rules and boundaries at each transition; rituals; intergenerational legacies; and the transitions particular to contemporary African and diasporic family life.

2. Learning outcomes

#	Outcome	Assessed
LO1	Place a family within the life-cycle model and identify its transition	Case analysis
LO2	Distinguish normative from non-normative transitions and their clinical implications	Exam
LO3	Analyse horizontal and vertical stressors in a family	Case analysis
LO4	Design a transition plan with a family	Observed session
LO5	Use rituals and marking to consolidate change	Skills test
LO6	Adapt the model for migration, polygyny, fosterage and non-linear trajectories	Essay
LO7	Work with loss and unfinished transitions	Reflective portfolio

3. Assessment

Component	Weight	Due
Life-cycle case analysis (2,000 words)	30%	Week 7
Transition plan observed session (40 min)	30%	Week 11

Component	Weight	Due
Essay on cultural adaptation (1,500 words)	20%	Week 13
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why transitions produce symptoms	Transition timeline
2	The six stages and their tasks	Stage identification
3	Normative and non-normative transitions	Stressor mapping
4	Horizontal and vertical stressors	Genogram + timeline
5	Stage 1 2: leaving home and joining	Case analysis
6	Stage 3: young children and the couple's reorganisation	Case analysis
7	**Case analysis due**; Stage 4: adolescents	Family session practice
8	Stage 5: launching and the empty nest	Couple re-negotiation
9	Stage 6: later life, caregiving and loss	Caregiver family meeting
10	Migration and displacement as family transitions	Migration genogram
11	**Transition plan observed**	Assessed
12	Divorce, remarriage and blended families	Structural adaptation
13	**Essay due**; births, deaths and unfinished transitions	Ritual design
14	Rituals, anniversaries and marking	Ritual design lab
15	Integration: the family in time	Case presentations

Week 1 Why transitions produce symptoms

The thesis. Symptoms appear at points of transition because the family's existing organisation is no longer adequate to the new demands, and the renegotiation has not yet happened.

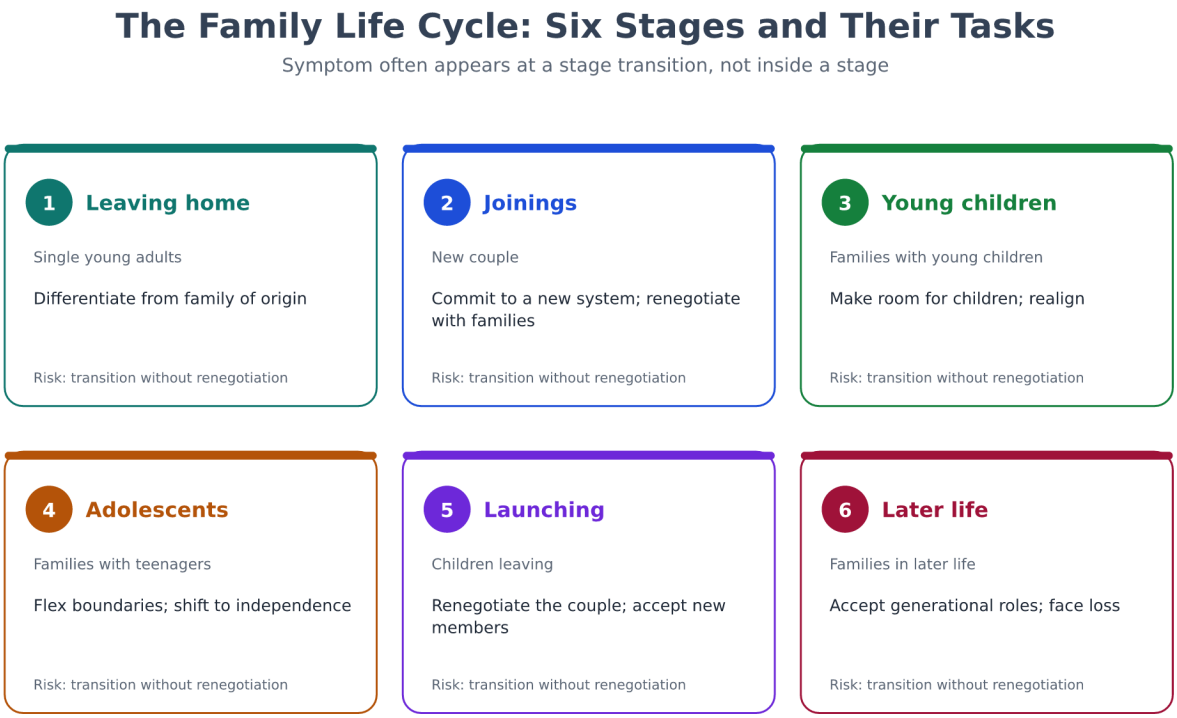


Figure 1.1 Six stages of the family life cycle and their tasks

Three mechanisms.

1. **Role vacuum.** A role disappears (the working mother, the eldest child at home) and no one has been assigned the new one.
2. **Boundary mismatch.** A boundary that suited the previous stage is now too rigid or too diffuse (parents who cannot let an adolescent have privacy; an adolescent who has become the mother's confidante).
3. **Unfinished business.** The previous transition was never completed (a father who never really left home; a family that never mourned a stillbirth), so the new one cannot be navigated.

Clinical question. "What changed in the last 6 18 months?" almost always produces the precipitant.

> **PRACTICAL**
> Draw a timeline of your family of origin marking moves, births, deaths, marriages, migrations, and job changes. Mark where symptoms or conflicts appeared. Note the pattern.

Week 2 The six stages

Stage	Emotional task	Second-order change required
1. Leaving home	Accept emotional and financial responsibility	Differentiate from family of origin; develop intimate peer relationships
2. Joining	Commit to a new system	Form a couple; realign relationships with extended families
3. Young children	Accept new members	Adjust the couple system; co-parent; realign with grandparents
4. Adolescents	Increase flexibility	Shift parent child boundaries; refocus on midlife marriage and careers
5. Launching	Accept many entries and exits	Renegotiate the marriage as a dyad; include in-laws and grandchildren
6. Later life	Accept shifting roles	Deal with loss, disability, and the next generation's centrality

Two warnings. The model is **descriptive of a particular Western, middle-class trajectory**. And the stages overlap: in many families, launching and caregiving occur simultaneously, children return home, and grandparents raise grandchildren.

Week 3 Normative and non-normative transitions

Normative transitions are expected: marriage, first child, school entry, adolescence, launching, retirement, death of elderly parents.

Non-normative transitions are unexpected or off-time: divorce, a child's disability, a death of a young parent, migration, redundancy, imprisonment, a pandemic, infertility, a child born outside marriage.

Why the distinction matters. Normative transitions have cultural scripts rituals, advice, role models. Non-normative transitions usually do not, which leaves families improvising under stress, often with shame.

Clinical implication. For non-normative transitions, part of the work is script-making: helping the family construct a way to do something no one they know has done.

> **PRACTICAL**
> Take a non-normative transition (e.g. a child's diagnosis). List the scripts that exist, the scripts that do not, and what the family has had to invent.

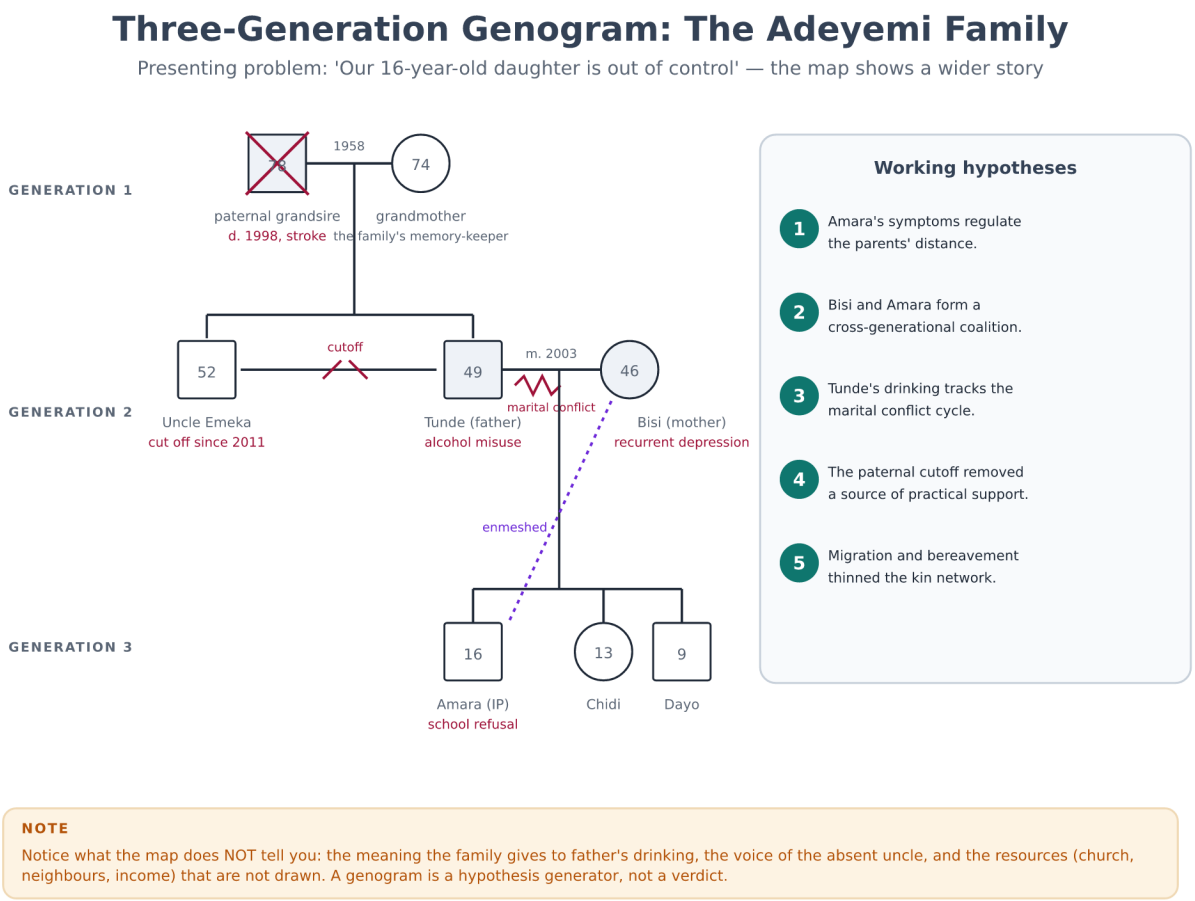
Week 4 Horizontal and vertical stressors

Horizontal stressors events moving through time: life-cycle transitions, illness, accidents, migration, poverty, discrimination.

Vertical stressors legacies handed down: family myths, expectations, secrets, taboos, ethnic and religious beliefs, scripts about gender and success.

The clinical insight. A family can usually manage horizontal stressors or vertical ones; the crisis comes when both arrive at once.

Example: A family facing redundancy (horizontal) in which the father's family has a myth that "a man who cannot provide is worthless" (vertical). The financial problem is manageable; the collision is not.



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 4.1 A genogram as a map of vertical stressors

Week 5 Leaving home and joining

Leaving home. The task is differentiation, not departure. A young adult may live at home and be differentiated, or live abroad and be fused. The clinical markers: can they disagree with a parent without catastrophe? Can they ask for help without regression?

Joining. The couple must build a "we" while remaining connected to two families of origin. The most common failure is a boundary problem: a spouse who remains more loyal to a parent than to the partner.

Assessment questions.

- "When you disagree with your mother, what happens?"
- "Who did you consult first about this decision?"
- "How often do you see each family? Who decided that?"
- "Whose opinion matters most in this marriage?"

Week 6 Young children

The reorganisation. Time, sleep, money, division of labour, sexuality, identity, and the relationship with grandparents all change at once. Research consistently shows a dip in relationship satisfaction after the first child, and that the quality of the co-parenting alliance predicts child outcomes more strongly than conflict.

The clinical presentations.

1. **The couple disappears into parents.** No couple time, no sexuality, no conversation that is not about the child.
2. **The grandmother becomes a third parent.** A cross-generational coalition forms; the father becomes peripheral.
3. **The child becomes the regulator.** A toddler's behaviour organises the household's emotional life.

Intervention. Protect the couple subsystem (time, privacy, an agreement about the division of labour); clarify the parental subsystem (a united front, agreed rules); and negotiate explicitly with grandparents about their role.

Week 7 Adolescents

The family task is to increase flexibility: more autonomy with more accountability; less control and more influence; boundaries that permit privacy without permitting secrecy.

Two failure patterns.

Pattern	What it looks like	Consequence
Escalation	Parents tighten as the adolescent pushes	Conflict, running away, secrecy, risk
Abdication	Parents withdraw to avoid conflict	Unsupervised risk, no structure, anxiety

The middle path is negotiation with limits: clear non-negotiables (safety), negotiable domains (time, phone, clothes), and consequences agreed in advance.

Also at this stage: the parents' own midlife renegotiation, which often appears as an "adolescent problem" in the referral.

Week 8 Launching and the empty nest

The task is to renegotiate the marriage as a dyad, and to accept new members and exits (partners, grandchildren, returns home).

The common presentations.

- **The couple who have nothing left to say.** They organised everything around the children; now there is silence.
- **The parent who cannot launch.** A mother who sabotages the child's independence because her role depends on being needed.
- **The child who will not leave.** Economic dependency, anxiety, or a parent's covert invitation.

Intervention. Rebuild the couple relationship deliberately; address the parent's identity and loss; and set a graduated plan for independence that neither humiliates the young adult nor abandons them.

Week 9 Later life

The tasks. Accept generational role changes; deal with loss (parents, siblings, spouse, friends); support the next generation without taking over; prepare for disability and death; and make meaning of a life.

Clinical presentations. Caregiving disputes among siblings; a widowed parent's new relationship; the "responsible child" who carries everything; inheritance conflict; a grandparent raising grandchildren.

Intervention. Make the loyalty ledger explicit (who has given what, and what is owed); negotiate caregiving tasks concretely; and address the unspoken grief that usually sits under the conflict.

The dual-process frame for grief (Figure 22 in MFT 712) helps here: restoration-oriented activity (tasks, roles, legal arrangements) is not avoidance; it is part of grieving.

Week 10 Migration and displacement

Migration is a family transition with all the features of a life-cycle stage and none of the rituals.

Phase	Family task	Common difficulty
Decision	Who goes, who stays, who pays	Coercion, debt, secrecy
Separation	Maintain attachment across distance	Children left with relatives; marital drift
Arrival	Establish a household	Status loss, housing, language

Phase	Family task	Common difficulty
Reunification	Re-form as a family	Children who do not know the parent; new authority conflicts
Settlement	Integrate without dissolving identity	Second-generation conflict; "you are not like us"

The "astronaut" and "parachute" patterns. A parent working abroad while the family remains; a child sent abroad for education. Both produce high achievement and high loneliness, and often a "satellite child" who is expected to justify the family's sacrifice.

Clinical questions. Who decided? Who was left? What was lost (status, language, community, identity)? What does the family owe the ones who stayed?

Week 11 Observed transition plan session

Forty minutes with a role-played family at a transition (launching, caregiving, migration reunification). Assessed on: accurate location of the transition; renegotiation of roles and boundaries; inclusion of all members; and a concrete, reviewable plan.

Week 12 Divorce, remarriage and blended families

Divorce as a transition, not an ending. The spousal relationship ends; the parental relationship continues in a different form. The family's task is to reorganise across two households while protecting the children from conflict.

The stages. Decision telling the children practical reorganisation legal process emotional adjustment new equilibrium (often 2 4 years).

Remarriage and stepfamilies. A new structure with different tasks:

- Build the couple bond before the stepparent assumes authority.
- Negotiate household rules across two systems.
- Allow children to love both households without disloyalty.
- Expect 4 7 years to integrate, not months.
- Address the "insider/outsider" split explicitly.

Clinical error. Treating the stepfamily as a nuclear family that started late, and applying a nuclear family timetable.

Week 13 Births, deaths and unfinished transitions

Unfinished transitions are those that were never marked, mourned or renegotiated: a stillbirth nobody mentions, a migration that was supposed to be temporary, a divorce that was never explained to the children, a death the family does not discuss.

Signs. Anniversary reactions, a name that cannot be spoken, a child named after the dead, a room kept unchanged, a recurring argument with no present cause.

Intervention. Name the unfinished transition gently; help the family construct a way to mark it; and allow the grief that the avoidance was managing.

Week 14 Rituals

Rituals consolidate transitions. They mark the boundary between the old state and the new, assign new roles, and connect the family to its wider culture.

Types. Cultural and religious rites (naming, marriage, burial); family rituals (Sunday meals, holidays, anniversaries); and therapeutic rituals (a letter, a ceremony, a shared object, a family meeting).

Designing a therapeutic ritual.

1. **Purpose.** What transition needs marking?
2. **Participants.** Who must be present, and who must be absent?
3. **Symbol.** What object, place, word or action carries the meaning?
4. **Structure.** Beginning, middle, end.
5. **Follow-up.** How will the change be maintained?

Caution. Respect the family's religious and cultural boundaries; do not invent rites that displace their own; and never use a ritual to bypass a necessary conversation.

> **PRACTICAL**
> Design a ritual for a family that has never marked a bereavement. Present it in five minutes.

Week 15 Integration: the family in time

The life-cycle lens gives you three questions that belong in every assessment:

1. **Where is this family in time?** (Stage, and the transition in progress.)
2. **What must be renegotiated?** (Roles, boundaries, loyalties, rules.)
3. **What has not been finished?** (Losses, transitions, legacies.)

Ask them in the first session and you will rarely be short of a focus.

Glossary

Horizontal stressors events and transitions moving through time. **Vertical stressors** legacies, myths and expectations transmitted across generations. **Normative transition** an expected, culturally scripted life event. **Non-normative transition** an unexpected or off-time event. **Second-order change** change in the system's organisation, not only in its behaviour. **Unfinished transition** a transition that was never marked or renegotiated. **Satellite child** a child sent abroad or away for education, carrying the family's hopes.

Self-assessment

Multiple choice

1. Symptoms most often appear: (a) randomly (b) at transitions requiring renegotiation (c) only in adolescence (d) only after trauma

2. Vertical stressors are: (a) events in time (b) legacies and myths across generations (c) financial problems (d) illnesses

3. The task of "leaving home" is: (a) departure (b) differentiation while remaining connected (c) financial independence only (d) marriage

4. The dip in couple satisfaction after a first child is: (a) rare (b) a well-documented common pattern (c) caused by the child (d) unavoidable

5. The two adolescent failure patterns are: (a) escalation and abdication (b) fusion and cutoff (c) pursuit and withdrawal (d) enmeshment and disengagement

6. Blended family integration typically takes: (a) months (b) 4 7 years (c) weeks (d) one year

7. A "satellite child" is: (a) a child in orbit (b) a child sent away for education carrying family hopes (c) an only child (d) a foster child

8. Unfinished transitions often present as: (a) clear grief (b) anniversary reactions and unspoken names (c) financial problems (d) delinquency

9. Therapeutic rituals should: (a) replace the family's own rites (b) respect and complement them (c) be religious (d) be avoided

10. The life-cycle model is best used as: (a) a timetable to enforce (b) a descriptive diagnostic lens (c) a legal standard (d) a diagnosis

Answer key

1 b 2 b 3 b 4 b 5 a 6 b 7 b 8 b 9 b 10 b

Short answer

1. Distinguish normative from non-normative transitions with two examples each.
2. Explain horizontal and vertical stressors with a case.
3. What must be renegotiated when a first child is born?
4. Name four features of migration as a family transition.
5. Give five elements of a therapeutic ritual.

Essay (choose one, 2,000 words)

1. "The life cycle is a Western timetable." Critically evaluate and adapt the model for Nigerian families.

2. Analyse a family's presenting problem as a transition problem, and show what changed in your treatment plan.
3. Discuss the role of rituals in consolidating therapeutic change.

Further reading

- Carter, B., & McGoldrick, M. (Eds.). (2005). The Expanded Family Life Cycle (3rd ed.). Allyn & Bacon.
- McGoldrick, M., Garcia-Preto, N., & Carter, B. (2016). The Multicultural Family Life Cycle. In The Expanding Family Life Cycle (5th ed.).
- Falicov, C. J. (2007). Working with transnational immigrants. Family Process, 46(2), 157 171.
- Walsh, F. (2016). Strengthening Family Resilience (3rd ed.). Guilford.

Next: MFT 720 Supervision and Consultation in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 20 OF 28

MFT 720

Supervision and Consultation in Therapy

15 teaching weeks 1 figure

SEMESTER THREE

1.71 units Prerequisite: MFT 706, 704

> Supervision is the profession's quality-control mechanism and its main method of training.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 720 Supervision and Consultation in Therapy

MSc Marriage and Family Therapy Semester Three 1.71 Units 15 teaching weeks Prerequisite: MFT 706, 704

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 720 Supervision and Consultation in Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	**Supervision and Consultation in Therapy**	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fifteen-week plan . . . 6

Week 1 What supervision is for . . . 6

Week 2 Developmental and competency models . . . 7

Week 3 Systemic and reflective models . . . 8

Week 4 Contracting . . . 8

Week 5 The supervisory alliance . . . 9

Week 6 Structure . . . 9

Week 7 Feedback . . . 10

Week 8 Live supervision and recording . . . 10

Week 9 Parallel process . . . 10

Week 10 Observed supervision session . . . 11

Week 11 Power, culture, gender and race . . . 11

Week 12 Conflict, incompetence and impairment . . . 11

Week 13 Evaluation and gatekeeping . . . 12

Week 14 Consultation to organisations . . . 12

Week 15 The supervisor you want to be . . . 13

Glossary . . . 13

Self-assessment . . . 13

- Multiple choice . . . 13
- Answer key . . . 14
- Short answer . . . 14
- Essay (choose one, 1,800 words) . . . 14

Further reading . . . 14

Course leader's summary. Supervision is the profession's quality-control mechanism and its main method of training. This course prepares you to receive supervision well and to give it competently with models, structure, feedback skills, attention to power, and the discipline of parallel process.

How to use this book

You are simultaneously a supervisee and a developing supervisor in this course. Each week you will analyse your own supervision (what worked, what did not, what you did not say) and practise supervising a peer. The two roles teach each other.

1. Course description

The course covers models of supervision (developmental, systemic, competency-based, reflective), the supervisory alliance, contracting and boundaries, live and recorded supervision, feedback, evaluation and gatekeeping, power and culture in supervision, parallel process, ethical and legal responsibilities, and consultation to organisations.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe and apply at least three supervision models	Exam
LO2	Contract clearly with a supervisee, including evaluation criteria	Contract document
LO3	Give specific, behavioural feedback that changes practice	Observed supervision
LO4	Identify and use parallel process	Case analysis
LO5	Manage power, culture and conflict in supervision	Reflective portfolio
LO6	Fulfil gatekeeping responsibilities ethically	Viva
LO7	Consult to a team or organisation	Consultation report

3. Assessment

Component	Weight	Due
Supervision contract and learning plan	15%	Week 4
Observed supervision session (40 min)	30%	Week 10
Case analysis of parallel process (1,500 words)	20%	Week 12
Consultation report (2,000 words)	25%	Week 14
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What supervision is for; supervision versus therapy versus mentoring	Role clarification
2	Models I: developmental and competency-based	Model application
3	Models II: systemic and reflective	Reflecting team in supervision
4	**Contract due**; contracting and boundaries	Contract negotiation
5	The supervisory alliance and rupture repair	Alliance repair rehearsal
6	Structure: agenda, case presentation formats, time	Structured case presentation
7	Feedback that changes practice	Feedback lab
8	Live supervision and recorded review	Live observation
9	Parallel process and the supervision triangle	Parallel process analysis
10	**Observed supervision session**	Assessed
11	Power, culture, gender and race in supervision	Power audit
12	**Parallel process analysis due**; conflict and incompetence	Difficult conversations
13	Evaluation, gatekeeping and fitness to practise	Evaluation interview
14	**Consultation report due**; consultation to organisations	Consultation simulation
15	Integration: the supervisor you want to be	Viva

Week 1 What supervision is for

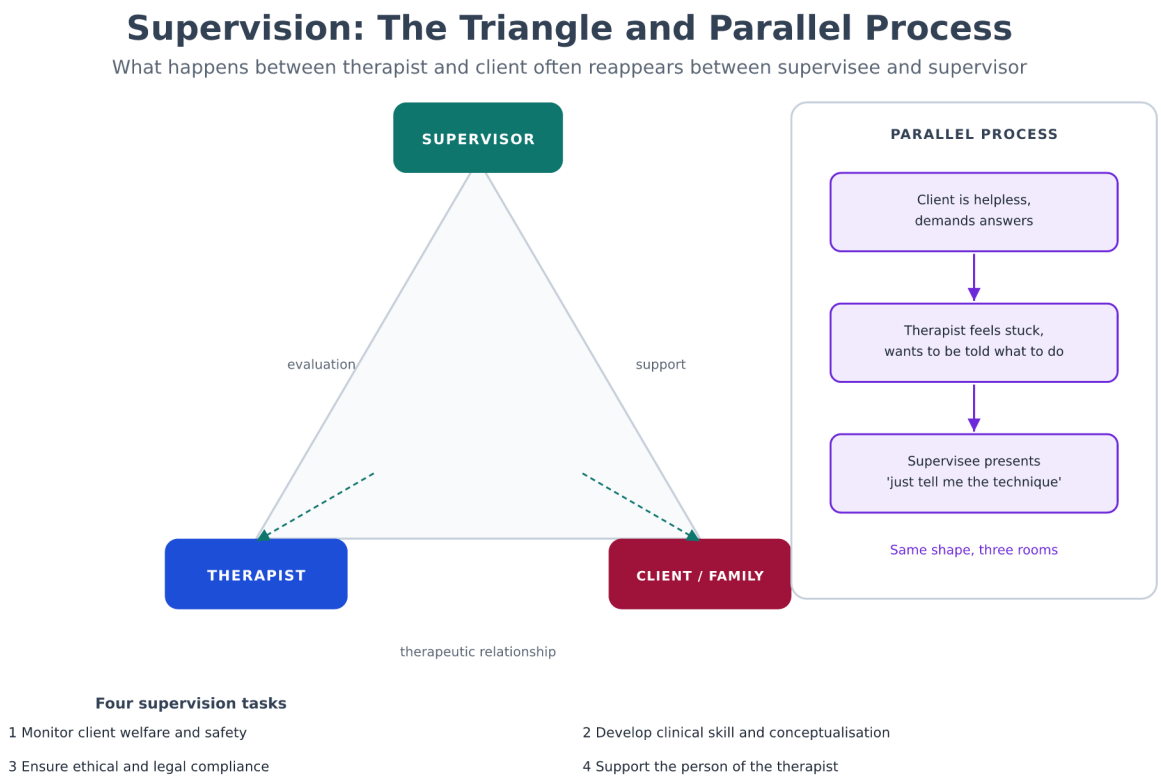


Figure 1.1 The supervision triangle and parallel process

Four functions (Kadushin, adapted).

1. **Clinical** ensuring the client receives good, safe care.
2. **Developmental** building the supervisee's competence.
3. **Supportive** sustaining the person doing difficult work.
4. **Gatekeeping** protecting the public and the profession.

What supervision is not. It is not therapy, although it may be therapeutic. It is not mentoring, which is about career. It is not line management, although it overlaps. Confusing these roles is the most common source of boundary difficulty.

The client is the third party in the room. Whatever the supervisee needs, the question that governs the supervision is: is this client safe and well served?

Week 2 Developmental and competency models

Developmental models (e.g. Stoltenberg) describe stages: the supervisee moves from dependence and anxiety, through dependence autonomy conflict, to conditional autonomy, to mastery. The supervisor's style must change with the stage: more structure early, more consultation later.

Stage	Supervisee needs	Supervisor stance
Novice	Structure, direct guidance, safety	Teaching, modelling, clear expectations
Advanced beginner	Autonomy with support	Coaching, prompting, joint formulation
Competent	Challenge, complexity	Consulting, debating, opening alternatives
Proficient	Peer-level dialogue	Collegial, focusing on the system and self

Competency-based models specify observable skills and rate them, using frameworks such as the UCL competence frameworks. Their strength is transparency and defensibility; their risk is reducing supervision to a checklist.

The best use of both. Use developmental stage to decide how much structure, and competencies to decide what to look for.

Week 3 Systemic and reflective models

Systemic supervision attends to the system of client therapist supervisor, and to the wider systems (agency, family, culture). The supervisor asks: what is the function of this presentation in this system, at this time?

Reflective supervision uses the supervisor's and supervisee's emotional responses as data, and models curiosity rather than expertise. Techniques include the reflecting team (in supervision), genograms of the case, and structured self-reflection.

Live supervision (behind a mirror, or in-room) gives immediate data. Formats: the phone-in, the "whispering" supervisor, the consultant who joins for ten minutes, and the reflecting team.

The advantage of live work is that it reveals what the therapist does, not what they remember doing. The disadvantage is anxiety, which must be managed explicitly in the contract.

Week 4 Contracting

A supervision contract covers:

1. Purpose and scope; who else is involved (agency, university).
2. Frequency, duration, location, cancellation.
3. Case presentation format and preparation expectations.
4. Recording and observation policy, including consent from clients.
5. Confidentiality and its limits (client safety, fitness to practise, legal).
6. Evaluation: criteria, timing, who sees it, and consequences.
7. Boundaries: contact between sessions, personal disclosure, dual relationships.
8. Procedures for disagreement, complaint and termination.

Negotiate, do not impose. The contract should include the supervisee's goals: "By the end of this placement, I want to be able to "

> **DOCUMENT (15%)**
> A supervision contract and learning plan for a first-placement supervisee.

Week 5 The supervisory alliance

Three components (parallel to the therapeutic alliance): goals, tasks, and bond.

Ruptures are inevitable: a supervisee feels judged; a supervisor feels ignored; a disagreement about a case becomes personal. The repair is the model.

Repair script.

> "I noticed you went quiet when I suggested you had missed the risk. Did I get that wrong, or did it land badly?" "What I intended was a question, not a verdict. Let me say it again differently."

The unspoken curriculum. Supervisees learn more from how you behave than from what you teach. If you avoid difficult feedback, they will avoid it with clients. If you are curious about your own mistakes, they will be curious about theirs.

Week 6 Structure

A 50-minute supervision session.

Minutes	Content
0 5	Check-in; agenda setting
5 15	Case 1: presentation, formulation, question
15 30	Case 1: exploration, recording review, options
30 40	Case 2 or a theme (risk, ethics, culture)
40 47	Learning points; tasks for the week
47 50	Administrative and evaluation notes

A case presentation format (five minutes, no notes).

1. Who is the family, and who is in the room?
2. What is the presenting problem, in their words?
3. What is my formulation and my uncertainty?
4. What did I do, and what happened?
5. What is my question for supervision?

The discipline matters: a supervisee who cannot present in five minutes is not yet thinking systemically.

Week 7 Feedback

Principles. Behavioural, specific, timely, owned, and limited to what can be acted on.

The four-step structure.

1. **What I observed** ("you asked three questions in a row after she cried").
2. **What it did** (to the client, or to me as observer) ("she stopped elaborating").
3. **What to keep** ("your opening was excellent you named the risk clearly").
4. **What to try** ("next time, try a reflection before the next question").

The banned practices. The sandwich (praise criticism praise), "you need to be more confident" (not behavioural), public criticism, and feedback given weeks after the event.

Receiving feedback is a skill too: listen, ask for an example, resist explaining, and decide what to use.

Week 8 Live supervision and recording

Consent. Families must consent to recording and to observation, in writing, with a clear explanation of who will watch and how long the recording is kept.

In-session formats.

- **Phone-in:** the supervisor calls in with one or two suggestions. Risk: the therapist becomes a puppet.
- **Reflecting team:** the team talks in front of the family (or behind a mirror). The therapist chooses what to bring back. Strong evidence and strong engagement effects.
- **Consultant joins:** a senior clinician joins for ten minutes and then leaves.

Debriefing. Always debrief the supervisee's experience first, then the clinical content. The order matters: the person before the problem.

Week 9 Parallel process

Definition. The pattern in the therapy reappears in the supervision: a helpless family produces a helpless therapist who produces a supervisor who takes over; a controlling family produces a therapist who complies and a supervisor who feels ignored.

How to use it.

1. Notice your own experience in supervision (boredom, irritation, the urge to solve).
2. Ask: is this mine, the supervisee's, or the system's?

3. Name it tentatively: "I notice I am doing a lot of the thinking here. I wonder whether that is also how it feels in the room."
4. Use it to generate a hypothesis about the family.

Caution. Parallel process is a hypothesis, not a law. Do not use it to explain away a supervisee's difficulty or your own poor supervision.

> **ANALYSIS (20%)**
> A 1,500-word analysis of a parallel process from your own supervision, with evidence and alternatives considered.

Week 10 Observed supervision session

Forty minutes, supervising a peer with a real (anonymised) case. Assessed on: contract and structure; case presentation discipline; formulation support; feedback quality; attention to the supervisee's experience; and clear learning tasks.

Week 11 Power, culture, gender and race

Power is structural. The supervisor evaluates, gatekeeps and holds institutional authority. It cannot be wished away; it can be made transparent.

Cultural dynamics. Differences in language, ethnicity, religion, gender, class and age shape what is said and what is withheld. A supervisee from a culture that does not openly disagree with elders may appear "unreflective" when they are being respectful.

Practical disciplines.

- Discuss difference explicitly at the start: "What should I know about how you prefer to receive feedback?"
- Invite disagreement in a form that is culturally possible: "What would you say if you disagreed?"
- Notice who speaks first, who is interrupted, and whose cases get the most time.
- Examine your own assumptions about competence.

Week 12 Conflict, incompetence and impairment

Conflict. Address early, privately, behaviourally. Distinguish disagreement about a case (healthy) from disagreement about values (harder) from a relationship rupture (needs repair).

Incompetence. Define it against the competencies, in writing, with a development plan and a review date. Most incompetence is developmental and remediable.

Impairment. When the supervisee's own difficulties affect client care (substance use, untreated trauma, exhaustion), the response is support plus boundaries, not silence. Silence is the most common failure of supervisors and the most damaging to clients.

The conversation. "I am concerned about you, and I am also concerned about your clients. Let us talk about both."

Week 13 Evaluation and gatekeeping

Evaluation criteria should be known in advance and behavioural.

Domain	Example indicator
Clinical competence	Can conduct a structured assessment and formulation
Professional conduct	Punctuality, record-keeping, boundaries
Reflective capacity	Uses feedback; identifies own contribution
Ethical practice	Recognises and escalates risk
Relational capacity	Builds alliances with all family members

Gatekeeping. If a supervisee is not safe, the supervisor must act, even when it is painful: extend supervision, restrict caseload, require therapy, or fail the placement with a documented rationale.

Failing well. Give clear reasons, a written plan, support, and a route to appeal. Document throughout, not only at the end.

Week 14 Consultation to organisations

The shift. Supervision develops a person; consultation helps a system. Your client is the team, the school, the clinic or the agency.

A consultation cycle.

1. **Contract:** who is the client, what is the question, who holds authority, what will be shared?
2. **Assess the system:** structure, communication, roles, alliances, and the presenting problem's function.
3. **Feedback:** present the systemic picture, with hypotheses rather than verdicts.
4. **Intervention:** a workshop, a reflecting team, a policy change, a case conference structure.
5. **Exit and evaluate:** what changed, and how will it be sustained?

Common organisational dynamics. The identified "difficult team member" who carries the team's anxiety; the "impossible case" that holds a conflict between services; the over-worked manager who mirrors the over-worked family therapist.

> **REPORT (25%)**
> A consultation report for a team or organisation, with a systemic assessment and a proposal.

Week 15 The supervisor you want to be

Write 700 words: your model of supervision; how you will handle power; what you will never do; the feedback you find hardest to give; and how you will know you are doing harm. Defend it in a viva.

Glossary

Competency-based supervision supervision organised around observable, rated skills. **Gatekeeping** the supervisor's responsibility to protect the public by preventing unsafe practice. **Live supervision** observation of therapy as it happens, with in-session input. **Parallel process** the reproduction of the therapy dynamic in the supervision. **Reflecting team** a group that discusses the case in front of the family, with the therapist choosing what to bring back. **Supervisory alliance** the goals, tasks and bond of the supervision relationship.

Self-assessment

Multiple choice

1. The client in supervision is: (a) the supervisee (b) the family receiving therapy (c) the agency (d) the university

2. Gatekeeping exists to: (a) protect the profession's reputation only (b) protect the public and the profession (c) increase pass rates (d) satisfy administrators

3. Parallel process is: (a) a proven law (b) a hypothesis about the reproduction of dynamics in supervision (c) transference (d) supervision of supervision

4. A good feedback statement is: (a) general and encouraging (b) behavioural, specific and owned (c) delayed (d) public

5. Live supervision requires: (a) no consent (b) written client consent and clear explanation (c) only supervisor consent (d) audio only

6. The sandwich technique is discouraged because: (a) it is too positive (b) it trains people to hear praise as a warning (c) it is fast (d) it is illegal

7. Developmental models suggest that novices need: (a) less structure (b) more structure and direct guidance (c) peer consultation only (d) no evaluation

8. Impairment in a supervisee should be met with: (a) silence (b) support plus clear boundaries (c) immediate dismissal (d) transfer without discussion

9. Consultation differs from supervision in that it: (a) develops a person (b) helps a system (c) evaluates students (d) is always clinical

10. A supervision contract should include: (a) only the times (b) purpose, frequency, confidentiality, evaluation and boundaries (c) the supervisor's fees only (d) the client's diagnosis

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Name the four functions of supervision.
2. Describe a five-minute case presentation format.
3. What are the four steps of behavioural feedback?
4. Give three ways power operates in supervision and one way to make it transparent.
5. What should a supervisor do when a supervisee is unsafe?

Essay (choose one, 1,800 words)

1. "Supervision is the profession's most important ethical practice." Discuss.
2. Analyse a parallel process from your own experience and what it revealed about the case.
3. Discuss the management of cultural difference in supervision, with examples.

Further reading

- Hawkins, P., & Shohet, R. (2012). Supervision in the Helping Professions (4th ed.). Open University Press.
- Stoltenberg, C. D., & McNeill, B. (2010). IDM Supervision (3rd ed.). Routledge.
- Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson.
- Andersen, T. (1991). The Reflecting Team. Norton.
- Pilling, S., & Fornells-Ambrojo, M. (2015). Supervision and Consultation Competence Frameworks. UCL.

Next: MFT 721 Research Methodology

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 21 OF 28

MFT 721

Research Methodology

14 teaching weeks 2 practicals 2 figures

SEMESTER THREE

1.71 units Prerequisite: MFT 705

> This is the advanced methods course that turns a proposal into a thesis.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 721 Research Methodology

MSc Marriage and Family Therapy Semester Three 1.71 Units 14 teaching weeks Prerequisite: MFT 705

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 721 Research Methodology. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	**Research Methodology**	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 Protocol and pre-registration . . . 6

Week 2 Design for relational questions . . . 7

Week 3 Sampling, power and feasibility . . . 8

Week 4 Data preparation . . . 8

Week 5 Regression . . . 8

Week 6 Multilevel models . . . 9

Week 7 Dyadic models . . . 9

Week 8 Qualitative analysis . . . 9

Week 9 Psychometrics . . . 10

Week 10 Systematic review basics . . . 11

Week 11 Single-case and small-N designs . . . 11

Week 12 Mixed methods integration . . . 12

Week 13 Robustness and pitfalls . . . 12

Week 14 Writing, publishing and defending . . . 13

Glossary . . . 13

Self-assessment . . . 13

- Multiple choice . . . 13
- Answer key . . . 14
- Short answer . . . 14
- Essay (choose one, 2,000 words) . . . 14

Further reading . . . 14

Course leader's summary. This is the advanced methods course that turns a proposal into a thesis. You will work with real data, choose and justify analytic strategies, defend your design against examiners, and learn to write findings that practitioners will actually read.

How to use this book

Every week's task contributes directly to your thesis. Bring your own dataset, or use the teaching dataset supplied with the course. Do not read statistics passively; run them, break them, and interpret them badly at least once.

1. Course description

The course covers advanced design (longitudinal, dyadic, single-case, participatory), analysis (regression, multilevel modelling, dyadic models, thematic and framework analysis, mixed-methods integration), measurement and psychometric evaluation, systematic reviewing, data management and reproducibility, writing for publication, and defence of a study.

2. Learning outcomes

#	Outcome	Assessed
LO1	Select and justify a design for a complex relational question	Thesis proposal
LO2	Conduct and interpret appropriate analyses for relational data	Analysis portfolio
LO3	Evaluate the psychometric quality of a measure in a new population	Psychometric report
LO4	Integrate quantitative and qualitative findings coherently	Mixed-methods report
LO5	Defend design decisions against alternative explanations	Viva
LO6	Write a publishable findings chapter	Thesis chapter
LO7	Manage data ethically, securely and reproducibly	Data plan and audit

3. Assessment

Component	Weight	Due
Analysis portfolio (4 exercises)	30%	Weeks 5, 8, 11, 13
Psychometric evaluation report (1,500 words)	20%	Week 9
Mixed-methods integration report (2,000 words)	20%	Week 12
Thesis methodology chapter + viva	30%	Week 15

4. The fourteen-week plan

Week	Topic	Task
1	From proposal to protocol; pre-registration and its limits	Protocol draft
2	Design for relational questions	Design decision log
3	Sampling, power and feasibility in clinical settings	Power calculation
4	Data preparation and the analysis plan	Data cleaning log
5	**Analysis 1:** regression and its assumptions	Run and interpret
6	Multilevel models for nested family data	Model comparison
7	Dyadic models: APIM in practice	APIM exercise
8	**Analysis 2:** thematic and framework analysis	Coding audit
9	Psychometrics: reliability, factor structure, invariance	Report
10	Systematic review and meta-analysis basics	Search and appraise
11	**Analysis 3:** single-case and small-N designs	SCED exercise
12	Mixed methods and integration	Joint display
13	**Analysis 4:** robustness, missing data, common pitfalls	Sensitivity analysis
14	Writing, publishing and defending	Chapter and viva prep

Week 1 Protocol and pre-registration

The protocol is the study's constitution: question, design, sample, measures, analysis plan, ethics, and timeline. Write it before you collect data; revise it with dated amendments if you must change course.

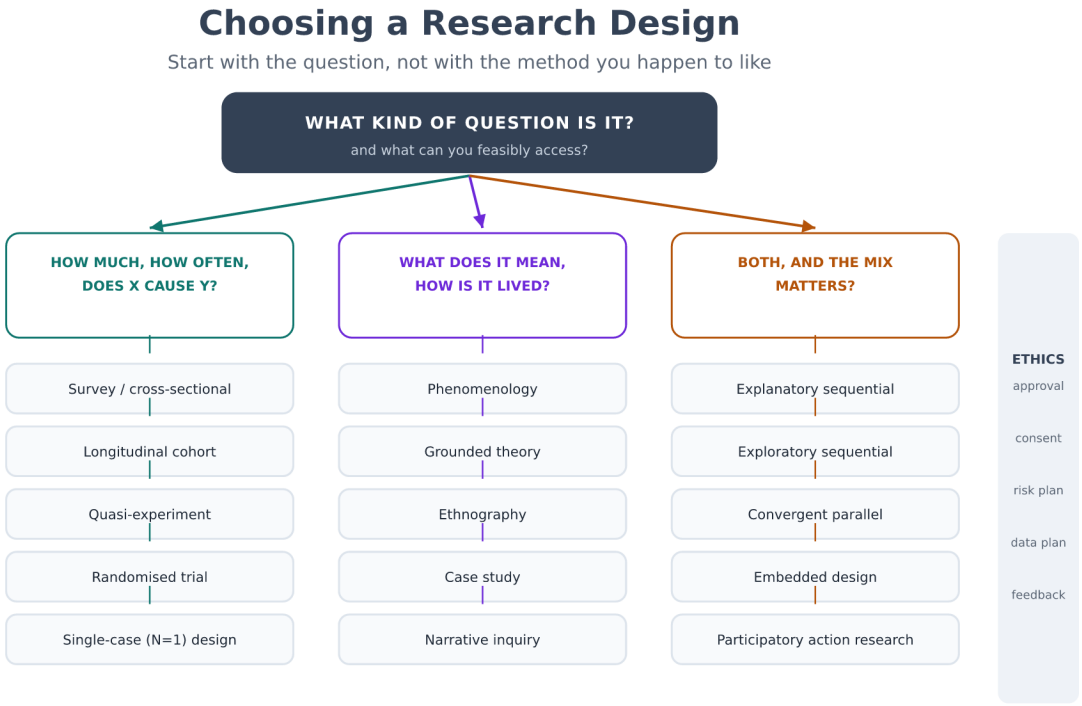
Pre-registration (e.g. on OSF) commits you publicly to a plan, which reduces the risk of unconscious analytical flexibility ("p-hacking"). It is standard in trials and increasingly expected elsewhere.

Honest limits. Exploratory qualitative work and community-based participatory research are often incompatible with rigid pre-registration. Declare your approach and be transparent about decisions made during the study.

A live analysis log one page per week, dated is the single most useful document in a thesis. It records what you did, what you decided, and why.

> **TASK**
> Draft a two-page protocol for your thesis. Include one paragraph on what you will do if recruitment is half of what you expect.

Week 2 Design for relational questions



NOTE

In MFT the unit of analysis is the relationship, not only the individual — which creates real statistical problems: partners are non-independent. Plan for it (dyadic data analysis, actor–partner models, or a design that samples one person per household).

Figure 2.1 Choosing a design from the question

Four designs especially suited to family questions.

Design	Question	Strength	Weakness
Longitudinal panel	Does X predict change in Y?	Temporal ordering	Attrition, cost
Dyadic (APIM)	How do partners affect each other?	Separates actor and partner effects	Requires both partners
Single-case experimental (SCED)	Does this intervention work for this family?	Clinical realism, causal inference within case	Generalisation
Participatory action research	What would improve this community's experience?	Validity, ethics, impact	Time, power negotiation

The design decision log. For each choice, record: the alternative considered, the reason for rejection, and the threat it leaves unresolved. Examiners reward this; it shows you know what your design cannot do.

Week 3 Sampling, power and feasibility

Power. Do the calculation before you collect (GPower, R's `pwr`, or simulation). For dyadic and multilevel designs, power depends on the number of clusters* (couples, families), not the number of individuals.

Practical realities in clinical settings.

- Referral rates: how many families does the clinic actually see per week?
- Eligibility: what proportion will meet your criteria?
- Consent: what proportion will agree? (Often 30 60% in clinical samples.)
- Attrition: assume 20 30% at follow-up.
- Seasonality: school terms, holidays, religious calendars.

The feasibility sentence. "To recruit 60 couples in six months, I need 4 eligible referrals per week with a 60% consent rate; the clinic receives 9 per week; two colleagues have agreed to refer."

Week 4 Data preparation

The unglamorous work that determines everything.

1. **Codebook** every variable, its label, its values, its missingness convention.
2. **Raw data frozen** never edit the original file after data entry.
3. **Cleaning log** every decision (a value of 99 recoded to missing; one duplicate removed).
4. **Missing data** describe the pattern; distinguish missing at random from missing not at random; consider multiple imputation rather than listwise deletion.
5. **Reverse scoring** check; a mis-scored subscale invalidates the results.
6. **Outliers** inspect, do not automatically delete.

> **TASK**
> Produce a codebook and cleaning log for your dataset (or the teaching dataset).

Week 5 Regression

What regression does. Estimates the association between predictors and an outcome, adjusting for other predictors, and expresses it in the outcome's units.

Assumptions to check. Linearity, independence of errors, homoscedasticity, normality of residuals (for inference), and no severe multicollinearity. Report the checks; do not just assert them.

Reading output. Coefficient (direction, size, units), standard error, confidence interval, p-value, R , and model fit. Report effect sizes with confidence intervals, not only p-values.

Clinical translation. "Each additional point on the criticism scale was associated with a 0.3-point lower satisfaction score (95% CI 0.5 to 0.1), holding depression constant." That sentence is worth more than a table of asterisks.

Week 6 Multilevel models

The problem. Individuals nested in couples, couples nested in therapists, repeated measures nested in people. Standard regression assumes independence and will produce confidence intervals that are too narrow.

The multilevel solution. Model the nesting explicitly:

- **Fixed effects** the average relationship across the sample.
- **Random effects** how intercepts (and slopes) vary across clusters.
- **ICC** the proportion of variance attributable to the cluster. A high ICC means the clustering matters greatly and cannot be ignored.

Model-building sequence. Null model (ICC) add level-1 predictors add level-2 predictors test random slopes compare with likelihood ratio tests or information criteria (AIC/BIC).

Report. Always report the ICC, the number of clusters, and the estimation method.

Week 7 Dyadic models

APIM in four numbers.

- **Actor effect (a):** my predictor on my outcome.
- **Partner effect (p):** my predictor on my partner's outcome.
- **Distinguishability:** whether partners are distinguishable (e.g. by gender or role) and whether effects can be constrained equal.
- **K:** the correlation between partners' residuals, which models the non-independence.

Interpretation example. In a study of 80 couples, criticism predicted lower satisfaction for the person giving it (actor effect) more strongly than for the person receiving it (partner effect). Clinical implication: the critic is also being harmed, which is a more useful reframe than "stop criticising your partner".

Software. R (`lme4`, `nlme`, `ape`), Mplus, SPSS MIXED.

Week 8 Qualitative analysis

Two workable approaches.

Approach	Logic	Output
Reflexive thematic analysis (Braun & Clarke)	Themes constructed from coding, with the analyst's role explicit	Rich thematic account

Approach	Logic	Output
Framework analysis (Ritchie & Spencer)	Matrix of cases themes, often for applied policy questions	Structured matrix, comparable across cases

Rigour in practice.

- Keep a **coding audit trail**: initial codes, revisions, discarded codes and why.
- Use **analytic memos** after each transcript.
- **Second-coder check** on at least 20% of data with a documented discussion of divergence.
- **Member reflection** where appropriate (not to validate themes, but to deepen them).
- **Reflexive journal** on your own reactions and assumptions.

The common error is presenting themes as "findings that emerged" from data, as though the analyst were not present. Themes are constructed; say so.

Week 9 Psychometrics



Figure 9.1 Validity, reliability and their limits

A psychometric evaluation of a measure in your population.

1. **Internal consistency** Cronbach's (and , which is more appropriate for congeneric items).

2. **Factor structure** exploratory or confirmatory factor analysis; report fit indices (CFI, TLI, RMSEA, SRMR).
3. **Construct validity** correlations with related and unrelated measures.
4. **Measurement invariance** configural, metric, scalar, across groups (gender, language, age).
5. **Responsiveness** change scores in a treated sample.

A local validation study is a perfectly good thesis, and often more useful to the field than another outcome study using an unvalidated instrument.

> **REPORT (20%)**
> A 1,500-word psychometric evaluation of one instrument, with recommendations.

Week 10 Systematic review basics

Steps. Register the protocol (PROSPERO) define the question (PICO) search systematically (at least three databases; record the strategy) screen (two reviewers; report agreement) extract data assess risk of bias (e.g. Cochrane RoB 2, or CASP for qualitative) synthesise (narrative or meta-analytic) assess certainty (GRADE).

Meta-analysis, briefly. Pool effect sizes with a random-effects model unless there is a justification for fixed effects; report heterogeneity (I ,); explore it with subgroup analysis or meta-regression if there are enough studies; assess publication bias with a funnel plot and Egger's test, understanding their limitations.

What examiners look for. A search strategy that could be reproduced; a bias assessment that is not perfunctory; and a conclusion proportionate to the evidence.

Week 11 Single-case and small-N designs

Why they matter in family therapy. Clinical samples are small, families are heterogeneous, and randomised trials are expensive. Single-case designs give causal inference within a case.

Core designs.

Design	Structure	Inference
A B	Baseline, then intervention	Weak (confounded by time)
A B A	Baseline, intervention, withdrawal	Stronger if reversal occurs
Multiple baseline across behaviours	Staggered introduction	Strong; no withdrawal needed
Multiple baseline across participants	Staggered across families	Strong; realistic in clinics

Analysis. Visual analysis (level, trend, variability) plus a quantitative index (e.g. Tau-U) and, ideally, systematic replication across cases.

Reporting standard. Follow the SCED reporting guidelines (e.g. SCRIBE), including baseline stability and treatment fidelity.

Week 12 Mixed methods integration

The integration question. What does the combination tell you that neither strand could alone?

Joint display format.

Quantitative finding	Qualitative finding	Convergence / divergence	Interpretation
Attendance predicted outcome (= .32)	Families described "being known" by the therapist as decisive	Divergence in mechanism	Attendance may be a proxy for alliance
Drop-out 34%	Families cited transport and work shifts	Convergence	Structural barriers, not motivation

Avoid the two common failures. Reporting two parallel studies with no integration; and using qualitative data merely to illustrate quantitative findings.

> **REPORT (20%)**
> A 2,000-word integration report with at least three joint display rows.

Week 13 Robustness and pitfalls

Pitfalls you will meet.

Pitfall	What it looks like	Fix
p-hacking	Many tests, reporting the significant ones	Pre-specify; adjust; report all
HARKing	Hypothesis stated after results known	Register the hypothesis
Overfitting	Model too complex for the sample	Simpler model; cross-validation
Ignoring clustering	Treating partners as independent	Multilevel or dyadic models
Selective reporting	Reporting one measure, ignoring another	Report all pre-specified outcomes
Causal language	"Caused" from cross-sectional data	Use associational language

Sensitivity analysis. Run your main analysis again with a different reasonable decision (e.g. excluding an outlier, using a different missing-data approach) and report whether conclusions change. If they do, say so.

Week 14 Writing, publishing and defending

The findings chapter. Present, do not interpret (that is the discussion's job). Structure by question. Report descriptive statistics before inferential. Use tables for numbers and prose for meaning.

The discussion. What you found how it compares with the literature mechanisms limitations (specific, not generic) implications for practice and policy future research.

Defending. Prepare for four questions: (1) Why this design? (2) What are the threats to validity and how did you address them? (3) Why these analyses? (4) What can and cannot be concluded? Rehearse short answers.

Publishing. Choose a journal that publishes practitioner research; follow the author guidelines exactly; expect a rejection and a revision; respond to reviewers point by point.

Glossary

APIM Actor Partner Interdependence Model. **Cluster** a grouping unit (couple, family, therapist) whose members are not independent. **GRADE** a system for rating the certainty of evidence. **ICC** intraclass correlation; the share of variance due to clustering. **Measurement invariance** equivalence of a measure's functioning across groups. **Pre-registration** public commitment to a study plan before data collection. **SCED** single-case experimental design. **Sensitivity analysis** re-analysis under different assumptions to test robustness.

Self-assessment

Multiple choice

1. Treating partners as independent observations primarily affects: (a) external validity (b) standard errors and Type I error (c) reliability (d) sampling

2. The ICC indicates: (a) internal consistency (b) the proportion of variance due to clustering (c) effect size (d) power

3. A multiple baseline design is strong because: (a) it requires withdrawal (b) staggered introduction controls for time (c) it uses large samples (d) it is qualitative

4. Joint displays are used to: (a) present descriptive statistics (b) integrate quantitative and qualitative findings (c) assess bias (d) calculate power

5. Pre-registration mainly reduces: (a) attrition (b) analytical flexibility and p-hacking (c) cost (d) sample size

6. Measurement invariance concerns: (a) whether a measure behaves equivalently across groups (b) internal consistency only (c) sample size (d) randomisation

7. A sensitivity analysis tests: (a) whether conclusions hold under different assumptions (b) the sample's normality (c) the coding frame (d) the ethics

8. HARKing means: (a) hypothesising after results are known (b) random sampling (c) qualitative coding (d) data entry

9. In framework analysis the output is typically: (a) a case theme matrix (b) a regression table (c) a factor loading matrix (d) a funnel plot

10. The most useful document in a thesis is: (a) the title page (b) a dated analysis log (c) the abstract (d) the reference list

Answer key

1 b 2 b 3 b 4 b 5 b 6 a 7 a 8 a 9 a 10 b

Short answer

1. Explain why clustering matters for standard errors.
2. Distinguish actor and partner effects with an example.
3. List five elements of a systematic review protocol.
4. What is the difference between a sensitivity analysis and a subgroup analysis?
5. Name four threats to validity in a pre-post clinic study.

Essay (choose one, 2,000 words)

1. Defend (or attack) the claim that randomised trials are the gold standard for family therapy research.
2. Present a design for a dyadic study of criticism and satisfaction, justifying every choice.
3. Critically appraise a systematic review of family interventions, focusing on method and certainty.

Further reading

- Kenny, D. A., Kashy, D. A., & Cook, W. L. (2006). Dyadic Data Analysis. Guilford.
- Hox, J., Moerbeek, M., & van de Schoot, R. (2017). Multilevel Analysis (3rd ed.). Routledge.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Higgins, J. P. T. et al. (Eds.). (2023). Cochrane Handbook for Systematic Reviews of Interventions.
- Kazdin, A. E. (2021). Single-Case Research Designs (3rd ed.). OUP.

Next: MFT 770 Master Thesis

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 23 OF 28

MFT 722

Professional Development and Self-Care

14 teaching weeks 1 practical 1 figure

ELECTIVES

1.71 units Prerequisite: MFT 706

> This is not a wellbeing add-on at the end of training.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 722 Professional Development and Self-Care

MSc Marriage and Family Therapy Elective 1.71 Units 14 teaching weeks Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 722 Professional Development and Self-Care. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	**Professional Development and Self-Care**	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 The person of the therapist . . . 6

Week 2 Burnout . . . 7

Week 3 Compassion fatigue and vicarious trauma . . . 7

Week 4 Boundaries . . . 8

Week 5 Personal therapy . . . 9

Week 6 Reflective practice . . . 9

Week 7 Supervision as a career-long practice . . . 9

Week 8 Continuing professional development . . . 10

Week 9 Career pathways . . . 10

Week 10 Money, fees and contracts . . . 11

Week 11 Private practice fundamentals . . . 11

Week 12 Professional identity and belonging . . . 11

Week 13 Practice readiness viva . . . 12

Week 14 CPD and career presentation . . . 12

Glossary . . . 12

Self-assessment . . . 12

- Multiple choice . . . 12
- Answer key . . . 13
- Short answer . . . 13
- Essay (choose one, 1,500 words) . . . 13

Further reading . . . 13

Course leader's summary. This is not a wellbeing add-on at the end of training. It is the course that decides whether you are still practising in fifteen years, and whether the practising is any good. It treats the therapist's own life as the primary instrument, and professional development as a discipline rather than an ambition.

How to use this book

Every week has a **professional** task and a **personal** one. Do both. The course is assessed largely on a portfolio, and a portfolio written retrospectively in the final week will be visible and weak. Keep a weekly entry; it takes eight minutes.

1. Course description

The course covers: the person of the therapist; burnout, compassion fatigue and vicarious trauma; boundaries and availability; personal therapy; reflective practice and supervision; continuing professional development; career pathways and private practice fundamentals; money, contracts and pricing; professional identity and belonging; and sustainable practice in resource-limited settings.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess your own burnout and vicarious trauma risk using structured tools	Portfolio
LO2	Design a personal and professional self-care plan that survives a busy month	Plan
LO3	Set and maintain boundaries with clients, colleagues and institutions	Viva
LO4	Plan continuing professional development against a competency framework	CPD plan
LO5	Explain the fundamentals of ethical private practice (contracts, fees, records)	Exam
LO6	Use reflective practice and personal therapy appropriately	Portfolio
LO7	Articulate a five-year professional trajectory	Presentation

3. Assessment

Component	Weight	Due
Reflective portfolio (10 entries)	35%	Fortnightly
Personal and professional sustainability plan (1,500 words)	25%	Week 10
Practice readiness viva (20 min)	25%	Week 13
CPD and career presentation (10 min)	15%	Week 14

4. The fourteen-week plan

Week	Topic	Task
1	The person of the therapist	Therapy autobiography (private)
2	Burnout: what it is and how it develops	Maslach self-assessment
3	Compassion fatigue and vicarious trauma	Symptom inventory
4	Boundaries: time, contact, disclosure, gifts	Boundary audit
5	Personal therapy and its place in training	Decide and plan
6	Reflective practice: models and habits	Weekly reflection structure
7	Supervision as a career-long practice	Supervision plan
8	CPD and competency frameworks	CPD gap analysis
9	Career pathways in MFT	Pathway map
10	**Sustainability plan due**; money, fees and contracts	Fee and contract design
11	Private practice fundamentals	Practice blueprint
12	Professional identity, community and belonging	Professional network plan
13	**Practice readiness viva**	Assessed
14	**CPD and career presentation**	Assessed

Week 1 The person of the therapist

The premise. You cannot separate the clinician from the person. Your history shapes what you notice, what you avoid, whom you like, and where you lose your balance.

The "therapy autobiography". Write, privately, about: your family of origin; the rules you absorbed; the losses; the roles you played; what you learned about conflict, intimacy, money and authority; and why you chose this work. Do not submit it. Do keep it, and revisit it in three years.

Common motivations and their risks.

Motivation	Strength	Risk
"I want to help people"	Compassion	Rescuing; over-functioning
"I want to understand my family"	Insight	Using clients to solve your own history
"People tell me I'm easy to talk to"	Alliance skill	Difficulty setting boundaries
"I survived something"	Credibility	Over-identifying; avoidance of certain material
"I want status/expertise"	Drive	Distance; difficulty with not-knowing

The professional requirement. Know your history well enough that you can tell the difference between your client's material and your own reaction. Personal therapy and supervision exist for this, not because something is wrong with you.

Week 2 Burnout

Definition (Maslach): a syndrome of **emotional exhaustion**, **depersonalisation/cynicism**, and **reduced sense of accomplishment**, arising from chronic occupational stress.

The six drivers (Maslach & Leiter) all of them organisational:

1. Workload
2. Control (or lack of it)
3. Reward (financial, social, intrinsic)
4. Community
5. Fairness
6. Values (mismatch between what you believe and what the system asks)

The clinically important point. Burnout is not a personal failing and cannot be solved by yoga. If the caseload is impossible, the supervision absent and the values compromised, self-care is a distraction from the real problem.

Self-assessment. Use the Maslach Burnout Inventory or the freely available Copenhagen Burnout Inventory. Score yourself honestly; write one paragraph on which of the six drivers is most active in your current situation.

Week 3 Compassion fatigue and vicarious trauma

Three distinct concepts.

Concept	Definition	Signature
Burnout	Exhaustion from workload	"I am too tired to care"
Compassion fatigue	Reduced empathy and satisfaction from caring	"I used to feel for people; now I don't"
Vicarious trauma	Changes to beliefs and inner world from exposure to others' trauma	"The world is not safe; people are dangerous"

Signs to monitor in yourself. Intrusion of client material; nightmares; avoidance of certain cases; cynicism; irritability at home; numbness; loss of hope; over-involvement; boundary erosion; sleep and appetite change; increased alcohol use.

Protective factors with evidence. Supervision that addresses the emotional impact; caseload diversity; control over scheduling; peer support; meaning-making; sleep and exercise; personal therapy; and organisational support.

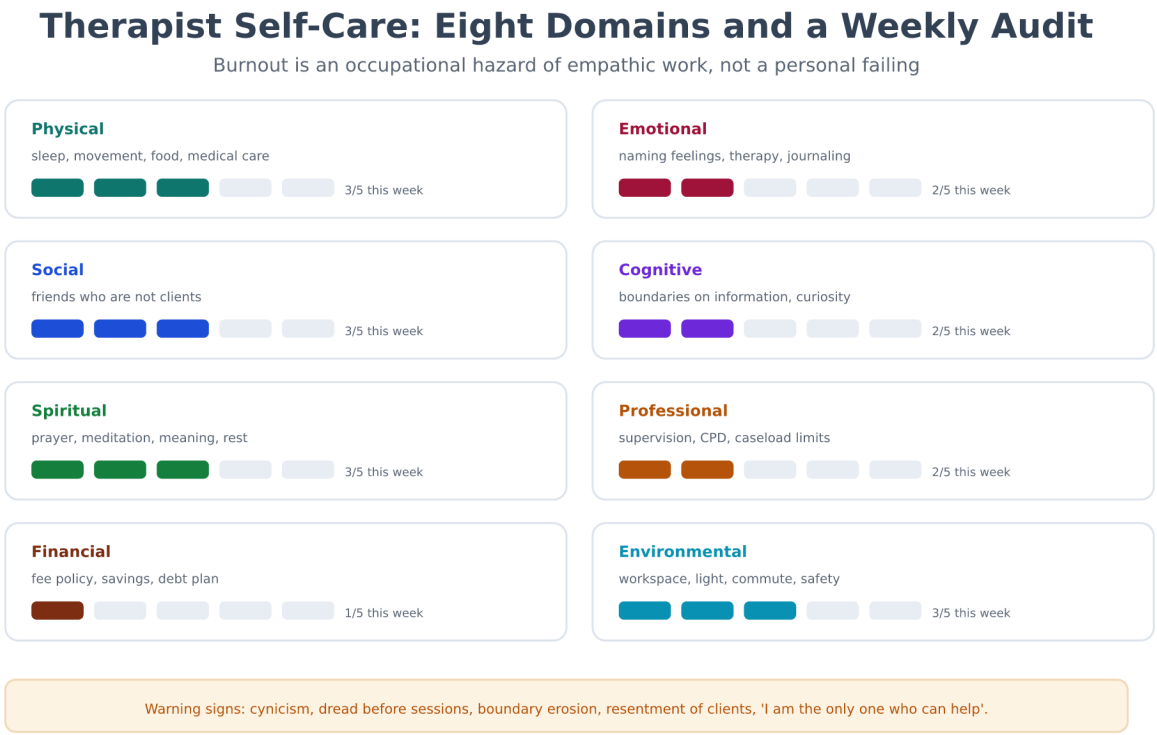


Figure 3.1 Self-care audit across eight domains

TASK

Complete the eight-domain audit. Rate each domain 0 5 for last week. Identify the lowest two and write one change for each that is small enough to happen on a Tuesday.

Week 4 Boundaries

The boundaries that matter most in early practice.

Boundary	The trap	A workable standard
Time	Sessions over-run; you arrive early, leave late	Fixed session length; 10 minutes between clients
Contact	Clients text at night; you reply	Written policy; emergency route; no personal number
Disclosure	You share to reassure	Disclose for the client's benefit, rarely, and briefly
Gifts	Refusing feels rude	Accept small cultural gifts; discuss significant ones
Social media	Client requests; you see their posts	Clear policy; never search for clients
Dual roles	Church, community, family ties	Disclose, discuss, refer if unmanageable
Emotional labour	You become the family's friend	Warmth plus frame; endings are explicit

The test. Not "was it kind?" but "whose need did it serve, and would I be comfortable explaining it in supervision?"

Week 5 Personal therapy

Requirements vary. Many training programmes require a minimum number of personal therapy hours. Treat the requirement as a floor, not a target.

What it is for. Not to become "fixed", but to know your patterns well enough to notice them in the room; to have a place to process work that is not your supervisor's job; and to experience being a client.

What to look for in a therapist. A different orientation from yours (to see another way of working); a recommendation from someone you trust; a clear contract; and someone you can be honest with.

The cost problem. Therapy is expensive, and trainees are often poorly paid. Options: reduced-fee schemes, group therapy, university counselling services, peer consultation groups, and budgeting it as a professional expense rather than a luxury.

Week 6 Reflective practice

Reflection is a method, not a mood. Use a structure.

The Gibbs cycle, applied to a session: description feelings evaluation analysis conclusion action plan.

Six questions to answer weekly.

1. What happened that surprised me?
2. What did I feel, and where in my body?
3. What did I avoid?
4. What did I do that worked?
5. What would I do differently?
6. What do I need to bring to supervision?

Writing. Keep one page per week. Do not write case details in a personal journal that is not secure; use initials or pseudonyms, and never store client-identifying material outside the clinical record.

Week 7 Supervision as a career-long practice

The norms. One hour of supervision for every five hours of clinical work is the common standard; more for complex cases and for trainees.

How to use supervision well.

- Arrive with an agenda and one clear question.
- Bring the case you least want to bring.
- Say what you felt, not only what you did.
- Ask for observation, not only opinion ("what would you have said?").
- Record what you will do differently, and report back next time.

Peer consultation groups. A complement, not a substitute: 3 5 clinicians, monthly, structured, with a confidentiality agreement and a rotating facilitator.

Week 8 Continuing professional development

Frameworks. Most professional bodies require annual CPD across domains: clinical practice, supervision, ethics, research, teaching, management. Map your year against the domains rather than collecting certificates.

A CPD plan that works.

1. Identify three gaps from your competency self-assessment.
2. For each, specify an activity, a date, and an evidence of completion.
3. Include one activity that stretches you (a new modality, a research skill, a leadership role).
4. Review every six months; adjust.

Beware CPD theatre. Ten webinars on topics you already know count for nothing. One supervised course in an area of genuine weakness counts for a great deal.

Week 9 Career pathways

Pathway	What it needs	What it gives	What it costs
Clinical specialist	Depth in a modality; supervision; certification	Expertise, referrals	Narrowing
Public sector clinician	Systems navigation; resilience	Security, breadth	Bureaucracy, heavy caseloads
Private practice	Business skills, capital, network	Autonomy, income potential	Isolation, income volatility
Academic/teaching	Research, publication, teaching	Influence, stability	Bureaucracy, distance from clinic
Supervision/consultation	Experience, accreditation	Leverage, variety	Responsibility for others' practice
NGO/humanitarian	Flexibility, languages, resilience	Meaning, breadth	Insecurity, exposure to trauma
Policy/advocacy	Writing, networks, patience	Scale of impact	Slowness, compromise

The honest question. Which of these fits the life you actually want — including your family, your health, and your finances — not only the career you imagined at 22?

Week 10 — Money, fees and contracts

Fees are clinical decisions. Set them deliberately; publish them; apply them consistently; review annually.

A fee policy covers: the fee; session length; payment method and timing; late cancellation; non-attendance; sliding scale criteria (decided in advance); annual review; and what happens to outstanding balances.

Contracts. Even a one-page agreement prevents most disputes: parties, purpose, confidentiality and limits, fees, cancellation, contact policy, complaints route, and signatures.

The uncomfortable truths.

- Charging is not exploitation; under-charging until you resent clients is.
- A sliding scale with no criteria becomes arbitrary and unfair.
- Chasing unpaid fees is a clinical conversation, not an administrative one.
- You will need to budget for tax, insurance, supervision, CPD and sick leave.

Week 11 — Private practice fundamentals

The blueprint. Location and room (privacy, sound, waiting area, accessibility); legal structure and registration; professional indemnity insurance; record-keeping and data protection; booking and payment systems; referral network; risk and emergency protocols; supervision arrangement; and a marketing approach that is ethical and honest.

Starting safely. Keep a part-time employed post while you build; take a business course or find a mentor; decide your niche only after 50 cases, not before; and never practise without supervision, however experienced you are.

Isolation is the occupational hazard. Build a peer group before you need one.

Week 12 — Professional identity and belonging

Belonging matters. Clinicians who are connected to a professional community report less burnout, better decision-making, and greater persistence.

How to build it. Join a professional association; attend one conference a year; contribute to a peer group; teach one workshop; write one short piece; mentor one junior colleague.

Identity in a contested field. Family therapy sits between psychology, psychiatry, social work and counselling. Be clear about who you are and what you offer, without disparaging

others: "I am a systemic therapist; my expertise is in relationships. I work alongside your psychiatrist, not instead of them."

Week 13 Practice readiness viva

Twenty minutes. Questions drawn from: boundary scenarios, burnout scenarios, ethical dilemmas, fee conversations, supervision difficulties, and career planning. Assessed on judgement, honesty, and the ability to name your own limits.

Week 14 CPD and career presentation

Ten minutes on where you are, where you are going, what you need, and what you will stop doing. Include one commitment to your own sustainability that is specific, dated, and verifiable.

Glossary

Burnout exhaustion, cynicism and reduced efficacy from chronic occupational stress. **Compassion fatigue** reduced empathy and satisfaction resulting from prolonged caring. **Compassion satisfaction** the positive counterpart: meaning and pleasure derived from helping. **CPD** continuing professional development. **Reflective practice** structured examination of one's own work and reactions. **Vicarious trauma** transformation of the therapist's inner world through exposure to others' trauma.

Self-assessment

Multiple choice

1. Burnout is primarily caused by: (a) personal weakness (b) chronic occupational stressors (c) too much supervision (d) client resistance

2. Vicarious trauma is characterised by: (a) tiredness (b) changes to beliefs and inner world (c) low fees (d) poor timekeeping

3. A workable contact boundary is: (a) giving your personal number (b) a written policy with an emergency route (c) replying at night (d) no policy

4. Personal therapy in training is best understood as: (a) a punishment (b) a professional requirement and an investment (c) unnecessary (d) a substitute for supervision

5. The recommended supervision ratio is roughly: (a) 1 hour per 20 client hours (b) 1 hour per 5 client hours (c) 1 hour per year (d) no standard

6. A useful CPD plan starts from: (a) available webinars (b) a competency gap analysis (c) cheap courses (d) colleagues' choices

7. Fee policies should be: (a) improvised (b) written, published and applied consistently (c) secret (d) identical everywhere

8. Peer consultation groups: (a) replace supervision (b) complement supervision (c) are unnecessary (d) are illegal

9. Reflection is best done: (a) once a year (b) weekly with a structure (c) only in crisis (d) verbally only

10. Private practice should ideally begin: (a) with no supervision (b) alongside employed work with supervision in place (c) with maximum debt (d) without insurance

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Name the three dimensions of burnout and the six organisational drivers.
2. Distinguish burnout, compassion fatigue and vicarious trauma.
3. List five boundaries and the standard you will use for each.
4. What are the six questions of a weekly reflection?
5. Outline the components of a fee policy.

Essay (choose one, 1,500 words)

1. "Self-care is a professional obligation, not a luxury." Discuss with reference to the evidence on burnout.
2. Design a sustainable practice model for a newly qualified family therapist in a low-resource setting.
3. Analyse your own family-of-origin patterns and their likely effect on your clinical work.

Further reading

- Norcross, J. C., & Guy, J. D. (2019). Leaving It at the Office (2nd ed.). Guilford.
- Figley, C. R. (Ed.). (1995). Compassion Fatigue. Brunner/Mazel.
- Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience. World Psychiatry, 15(2), 103 111.
- Skovholt, T. M., & Trotter-Mathison, M. (2014). The Resilient Practitioner (3rd ed.). Routledge.
- Bolton, G. (2018). Reflective Practice (5th ed.). Sage.

Next: MFT 723 Special Populations in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 24 OF 28

MFT 723

Special Populations in Therapy

ELECTIVES

1.71 units · Prerequisite: MFT 708, 714

> “"Special populations" is a compromise term.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0 · 2026 edition

MFT 723 — Special Populations in Therapy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 708, 714

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 723 — Special Populations in Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	**Special Populations in Therapy**	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 — Forced displacement . . . 6

Week 2 — Asylum-seeking families . . . 7

Week 3 — LGBTQ+ clients and their families . . . 7

Week 4 — Disability . . . 7

Week 5 — Chronic illness and palliative care . . . 8

Week 6 — Older adults . . . 8

Week 7 — Criminal justice involvement . . . 8

Week 8 — Uniformed services . . . 9

Week 9 — Single-parent and grandparent-headed households . . . 9

Week 10 — Infertility and childlessness . . . 9

Week 11 — Families affected by HIV . . . 10

Week 12 — Street-connected children and youth . . . 10

Week 13 — Intersectionality and service design . . . 10

Week 14 — Integration . . . 11

Glossary . . . 11

Self-assessment . . . 11

- Multiple choice . . . 11
- Answer key . . . 12
- Short answer . . . 12
- Essay (choose one, 2,000 words) . . . 12

Further reading . . . 12

Course leader's summary. "Special populations" is a compromise term. It names groups whom services tend to miss, misread or harm — and it trains the clinician to adapt the frame without lowering the standard of care.

How to use this book

Each week takes one population and works through four questions: what is the specific stressor, what does the evidence say helps, what must be adapted, and what should I not do? The aim is competence in adaptation, not expertise in every group.

1. Course description

The course covers families affected by forced displacement; LGBTQ+ clients and their families; people with disabilities and chronic illness; older adults; families involved with the criminal justice system; veterans and uniformed services; single-parent and grandparent-headed households; childless and infertile couples; families affected by HIV; and street-connected children.

Each carries a specific combination of stigma, legal precarity, service exclusion and clinical complexity that requires a specific adaptation of the systemic frame.

2. Learning outcomes

#	Outcome	Assessed
LO1	Identify the specific stressors and strengths of each population	Exam
LO2	Adapt assessment and intervention without diluting standards	Case study
LO3	Recognise and interrupt the ways services exclude these groups	Reflective portfolio
LO4	Work with interpreters, advocates and community organisations	Skills test
LO5	Address legal, safeguarding and ethical complexities	Viva
LO6	Evaluate the evidence for culturally adapted interventions	Essay
LO7	Design an accessible service pathway	Proposal

3. Assessment

Component	Weight	Due
Case study with adaptation plan (2,500 words)	40%	Week 9
Service pathway proposal (1,500 words)	25%	Week 12
Viva on legal and ethical complexity (20 min)	15%	Week 13
Examination	20%	Week 14

4. The fourteen-week plan

Week	Population	Adaptation focus
1	Families affected by forced displacement	Safety, loss, limbo
2	Refugees and asylum-seeking families	Legal precarity and the "second trauma"
3	LGBTQ+ clients and their families	Risk, disclosure, acceptance
4	Families with a member with a disability	Access and the social model
5	Chronic illness and palliative care families	Illness as a family member
6	Older adults and their families	Ageing, loss, elder abuse
7	Criminal justice involvement and imprisonment	Absence, stigma, re-entry
8	Families of uniformed services	Deployment, injury, silence
9	**Case study due**; single-parent and grandparent-headed households	Structure and load
10	Infertility, childlessness and reproductive loss	Stigma, grief, gender
11	Families affected by HIV	Stigma, secrecy, transmission anxiety
12	**Pathway proposal due**; street-connected children and youth	Safety, trust, systems
13	**Viva**; intersectionality and service design	Integrated planning
14	Integration and examination	Review

Week 1 — Forced displacement

The specific stressors. Loss (home, status, language, network, identity); prolonged uncertainty; trauma before, during and after flight; family separation; role reversal; and the "limbo" of waiting for legal status.

The clinical tasks. Safety and basic needs first; normalising distress without pathologising; restoring roles; supporting the family's own rituals and faith; and addressing the grief of the life left behind — often unmourned because the family is still in survival mode.

Adaptation.

- Do not require a full trauma narrative in early sessions.
- Use practical problem-solving alongside emotional work; both are therapeutic.
- Involve community and faith organisations.
- Ask about the family members who are missing — literally and emotionally.

Week 2 — Asylum-seeking families

The "second trauma" is the legal process: interviews, credibility assessments, detention, dispersal, uncertainty, and the requirement to tell the worst story repeatedly to strangers.

Clinical realities.

- Status anxiety dominates every decision: whether to report violence, whether to seek medical care, whether to disclose sexuality.
- Children may be the family's cultural and linguistic bridge.
- Court and immigration reports may be requested; the report and the therapy must be kept distinct.

What helps. Concrete assistance with documentation and access; clear, honest explanation of what you will write and to whom; liaison with legal representatives (with consent); and a therapeutic space where the person is not required to justify their deserving.

Week 3 — LGBTQ+ clients and their families

Minority stress. Prejudice, expectation of rejection, internalised stigma and concealment produce chronic stress and elevated rates of anxiety, depression, self-harm and suicide.

Family work. Families often present in crisis after disclosure, with religious and cultural conflict. The work is to: create safety for the young person; hear the parents' fear without endorsing rejection; slow the timetable (rejection, like acceptance, is a process); and set limits on harm — verbal abuse, expulsion, forced "corrective" interventions.

What not to do.

- Do not promise to change the young person, or to change the family.
- Do not require the client to come out as a condition of care.
- Do not collude with secrecy that is unsafe, or with disclosure that is premature.
- Do not refer to "the lifestyle"; use the client's language.

The clinical stance. You are not there to adjudicate theology or politics. You are there to reduce harm, improve relationships where possible, and support the person's dignity.

Week 4 — Disability

The two models. Medical (the body is the problem) and social (society's barriers are the problem). Clinically, both matter.

What families need.

- Accurate information about the condition and its trajectory.
- Help with the burden of care and the sibling experience.
- Support for the parental relationship, which is often absorbed by the caregiving.

- Advocacy with schools, employers and health services.
- Attention to the disabled person's own voice and autonomy, which families often override in the name of protection.

Adaptation. Accessible rooms; longer sessions; communication support; written materials in accessible formats; and a willingness to conduct sessions in the home when necessary.

Week 5 — Chronic illness and palliative care

The illness as a third member of the family. It takes a role, demands attention, reorganises routines and relationships.

Clinical tasks. Rebalance the couple relationship when one partner becomes a carer; address the children's understanding and fears; manage the family's beliefs about illness and fate; support decision-making about treatment; and prepare for death without abandoning hope.

Palliative and end-of-life work. Advance care planning; legacies; unfinished business; the family's conflicts about treatment; and bereavement support afterwards.

The clinician's own mortality. This work touches your own fears. Supervision is essential.

Week 6 — Older adults

Specific stressors. Retirement, bereavement, physical decline, loss of role, relocation, dependence, and the assumption by others that life is over.

Clinical presentations. Depression misdiagnosed as ageing; anxiety; cognitive change (which must be differentiated from delirium, depression, hearing loss and dementia); family conflict about care; elder abuse; and grief.

Adaptation. Slower pace; larger print; hearing support; sessions at home; involving the family in practical decisions; and taking the older person's goals as primary, not the family's convenience.

Elder abuse. Ask directly and separately: "Has anyone hurt you, taken your money, or stopped you seeing people?" Know your safeguarding route.

Week 7 — Criminal justice involvement

The absence. Imprisonment removes a person from the household while keeping them psychologically present. Children are often told a story; partners carry shame and practical burden.

Clinical tasks. Support the remaining household; address the children's understanding and stigma; prepare for release and re-entry; manage the couple's renegotiation; and address the family's history (which may include the behaviour that led to imprisonment).

Re-entry. The returning member often expects to resume an old role, while the family has reorganised without them. Naming this mismatch early prevents a second rupture.

Adaptation. Sessions in prison settings; rules about contact; careful confidentiality (the prison is not a confidential setting); and coordination with probation or social services.

Week 8 — Uniformed services

The specific culture. Stoicism, mission focus, hierarchy, and a strong norm against help-seeking.

Clinical presentations. Deployment and reintegration; moral injury; PTSD; substance use; domestic violence; and the family's secondary traumatisation.

Adaptation. Frame treatment in terms the culture respects (performance, mission readiness, the family's stability); involve the unit where appropriate; and address the couple's shift in roles during absence and return.

Week 9 — Single-parent and grandparent-headed households

The specific stressors. Role overload; financial strain; the absent parent's shadow; stigma; and children's questions.

Strengths. Many single-parent households function very well; treat the structure as a structure, not a deficit.

Grandparent-headed households. Grief (for the parent, often due to death or incapacity); the generation gap; legal status (custody, maintenance); health limitations; and the children's loyalty conflicts.

Adaptation. Practical support first; a clear parenting plan; support for the child's contact with the absent parent where safe; and attention to the carer's own health.

Week 10 — Infertility and childlessness

The cultural weight. In many settings, childlessness carries severe social stigma, blamed disproportionately on women, with consequences for marriage stability and family standing.

Clinical themes. Grief (often disenfranchised); shame; blame and the couple's conflict; pressure from extended family; religious meaning; treatment decision-making and its cost; and the couple's sexual relationship, which becomes medicalised.

The clinical stance. Do not assume that parenthood is the goal for every couple. Ask what a good life looks like for them. Name the social pressure as external, not personal failure.

Week 11 — Families affected by HIV

The clinical realities. Stigma and secrecy; disclosure decisions (to partner, children, family, church); medication adherence; transmission anxiety; bereavement and orphanhood; and the intersection with poverty and gender inequality.

Disclosure work. Help the person decide who, when and how; rehearse; prepare for different responses; and ensure the children's questions are answered at their developmental level.

Adaptation. Confidentiality is paramount in small communities; coordinate with HIV services carefully; address the family's beliefs about illness and blame; and support the "worried well" partner as well.

Week 12 — Street-connected children and youth

The specific realities. Survival needs first (food, safety, shelter); mistrust of adults and institutions; peer networks that function as family; substance use; exposure to violence and exploitation; and legal vulnerability.

What helps. Outreach and continuity; a trusted adult who does not disappear; harm reduction; family reunification work only where it is safe; and collaboration with community organisations, not only clinics.

What harms. Coercive removal; moralising; short-term projects that end abruptly; and treating the peer group as pathology when it is often the only protection available.

Week 13 — Intersectionality and service design

The design questions.

1. Who does not come to our service, and why?
2. What are the access barriers — cost, transport, language, hours, stigma, documentation, disability?
3. Who is the first point of contact, and are they trained?
4. What does the physical environment communicate?
5. What happens when a client needs an interpreter, a wheelchair, a child-friendly space, or a private entrance?
6. How do we measure whether these groups are actually being served?

A pathway that works typically includes: community outreach; a named link worker; flexible appointments; interpreters; transport support; plain-language materials; and a feedback loop with the communities served.

> **PROPOSAL (25%)**
> A service pathway for one under-served population, with access analysis, staffing, and outcome measures.

Week 14 — Integration

The generalizable skills. Ask rather than assume; adapt the frame, not the standard; work with community organisations; attend to legal precarity; protect confidentiality in small communities; and examine your own discomfort.

The generalizable caution. "Special population" can become a way of othering. Every family is specific; the skill is in the specificity, not in the label.

Glossary

Asylum seeker — a person seeking protection who has not yet been granted refugee status. **Disenfranchised grief** — grief that is not socially recognised or supported. **Minority stress** — chronic stress from stigma, prejudice and concealment. **Moral injury** — psychological distress from acting against one's values in service. **Social model of disability** — disability located in societal barriers rather than the body.

Self-assessment

Multiple choice

1. The "second trauma" for asylum seekers refers to: (a) the original event (b) the legal process (c) the journey (d) therapy

2. Family work after a young person's coming out should first: (a) change the young person (b) create safety and hear the parents' fear (c) enforce acceptance (d) require celibacy

3. The social model of disability locates disability in: (a) the body (b) societal barriers (c) genetics (d) the family

4. Elder abuse should be: (a) assumed unlikely (b) asked about directly and separately (c) ignored (d) reported without consent always

5. On release from prison, families commonly: (a) resume old roles easily (b) have reorganised without the returning member (c) refuse contact (d) need no support

6. Childlessness in many settings is: (a) neutral (b) severely stigmatised and blamed on women (c) rare (d) a legal matter

7. For street-connected youth, the first priority is: (a) family therapy (b) survival needs and a trusted adult (c) diagnosis (d) schooling only

8. Minority stress explains elevated rates of: (a) income (b) anxiety, depression and self-harm (c) physical fitness (d) employment

9. Effective pathways for under-served groups begin with: (a) more clinics (b) access analysis and community outreach (c) longer waiting lists (d) more diagnosis

10. The risk of the term "special populations" is: (a) inaccuracy (b) othering and stereotyping (c) legality (d) cost

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name four specific stressors of forced displacement and one clinical task for each.
2. What are three "do nots" in family work after a coming-out disclosure?
3. How does the social model of disability change your assessment?
4. What must a service pathway include for under-served populations?
5. Why is confidentiality harder in small communities, and what do you do about it?

Essay (choose one, 2,000 words)

1. "Adaptation without dilution." Discuss with reference to one special population.
2. Critically evaluate the evidence for culturally adapted family interventions.
3. Design an accessible family therapy service for one excluded group in your context.

Further reading

- Falicov, C. J. (2014). Latino Families in Therapy (2nd ed.). Guilford.
- Papadopoulos, R. K. (Ed.). (2002). Therapeutic Care for Refugees. Karnac.
- Laing, L., & Topping, K. (2019). Family Therapy with Older Adults. Routledge.
- Lev, A. I., & Sennott, S. L. (2019). Gender and Sexuality in Family Therapy. Routledge.
- Miller, G. (2011). Management of Alcohol and Drug Problems. (For harm-reduction principles in outreach work.)

Next: MFT 724 — Family Law and Advocacy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 25 OF 28

MFT 724

Family Law and Advocacy

14 teaching weeks 2 figures

ELECTIVES

1.71 units Prerequisite: MFT 704

> Family therapists work at the edge of the legal system: custody disputes, protection proceedings, divorce, maintenance, guardianship.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0
2026 edition

MFT 724 Family Law and Advocacy

MSc Marriage and Family Therapy Elective 1.71 Units 14 teaching weeks Prerequisite: MFT 704

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 724 Family Law and Advocacy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	**Family Law and Advocacy**	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 Why legal literacy matters . . . 6

Week 2 Four marriage systems . . . 7

Week 3 Divorce and dissolution . . . 8

Week 4 Custody, access and the welfare principle . . . 8

Week 5 Maintenance and financial provision . . . 9

Week 6 Child rights and protection . . . 9

Week 7 Domestic violence law . . . 9

Week 8 Mediation and alternative dispute resolution . . . 10

Week 9 Reports and court . . . 10

Week 10 Expert versus treating roles . . . 10

Week 11 Working with lawyers and courts . . . 11

Week 12 Mediation skills assessment . . . 12

Week 13 Advocacy . . . 12

Week 14 Advocacy proposal and integration . . . 12

Glossary . . . 12

Self-assessment . . . 13

- Multiple choice . . . 13
- Answer key . . . 13
- Short answer . . . 13
- Essay (choose one, 2,000 words) . . . 13

Further reading . . . 14

Course leader's summary. Family therapists work at the edge of the legal system: custody disputes, protection proceedings, divorce, maintenance, guardianship. This course gives you the legal literacy to be useful, the boundaries to stay in your lane, and the advocacy skills to change the systems your clients pass through.

How to use this book

This is orientation, not legal advice. Statutes, case law and state practice vary and change. Every week names the questions you should ask a lawyer, not the answers you should give a client. The final weeks turn from individual cases to systemic advocacy.

1. Course description

The course covers: the Nigerian marriage and family law framework (statutory, customary, Islamic and cohabitation); divorce and dissolution; custody, access and maintenance; the welfare principle; child rights and protection law; domestic violence legislation; mediation and alternative dispute resolution; report writing and expert testimony; the therapist's role in court; and advocacy for policy and service change.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the four marriage systems and their dissolution routes	Exam
LO2	Describe the legal framework for custody, maintenance and child protection	Exam
LO3	Distinguish therapy, mediation, expert assessment and legal advice	Viva
LO4	Write a defensible court-facing report within your competence	Report
LO5	Prepare a client for legal processes without becoming an advocate for a version of events	Case study
LO6	Conduct safe mediation or refer appropriately	Skills test
LO7	Design an advocacy campaign or policy proposal	Proposal

3. Assessment

Component	Weight	Due
Legal framework examination	25%	Week 7
Court-facing report (2,000 words)	30%	Week 10
Mediation skills assessment (30 min)	20%	Week 12
Advocacy proposal (1,500 words + presentation)	25%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why family therapists need legal literacy	Role boundary mapping
2	Four marriage systems	Case classification
3	Divorce and dissolution	Route mapping
4	Custody, access and the welfare principle	Custody scenario
5	Maintenance and financial provision	Calculation exercise
6	Child rights and protection law	Reporting scenario
7	**Examination**; domestic violence law	Protection order pathway
8	Mediation and ADR	Mediation demonstration
9	The therapist in court: reports and testimony	Report drafting
10	**Report due**; expert versus treating roles	Role conflict analysis
11	Working with lawyers and courts	Interprofessional simulation
12	**Mediation assessment**	Assessed
13	Advocacy: from case to policy	Campaign design
14	**Advocacy proposal**; integration	Presentations

Week 1 Why legal literacy matters

Three reasons.

1. **Your clients are inside legal processes.** Divorce, custody, maintenance, protection orders, immigration, inheritance. If you do not know the terrain, you cannot help them navigate it.
2. **You will be asked for opinions you should not give.** "Will I get custody?" is a legal question with a clinical component. Confusing them harms clients.
3. **You may be a witness or a report-writer.** The quality of your documentation determines whether your evidence is useful or damaging.

The boundary map.

Activity	Who does it	Your role
Legal advice	Lawyer	Refer; explain the difference
Mediation	Trained mediator	Conduct only if trained and if safe; otherwise refer
Therapeutic work	You	Your core role
Expert assessment (custody)	Independent expert	Generally not your own client

Activity	Who does it	Your role
Court testimony	Witness	Fact witness; opinion only within expertise
Advocacy	You, as a professional	Legitimate, within your competence and evidence

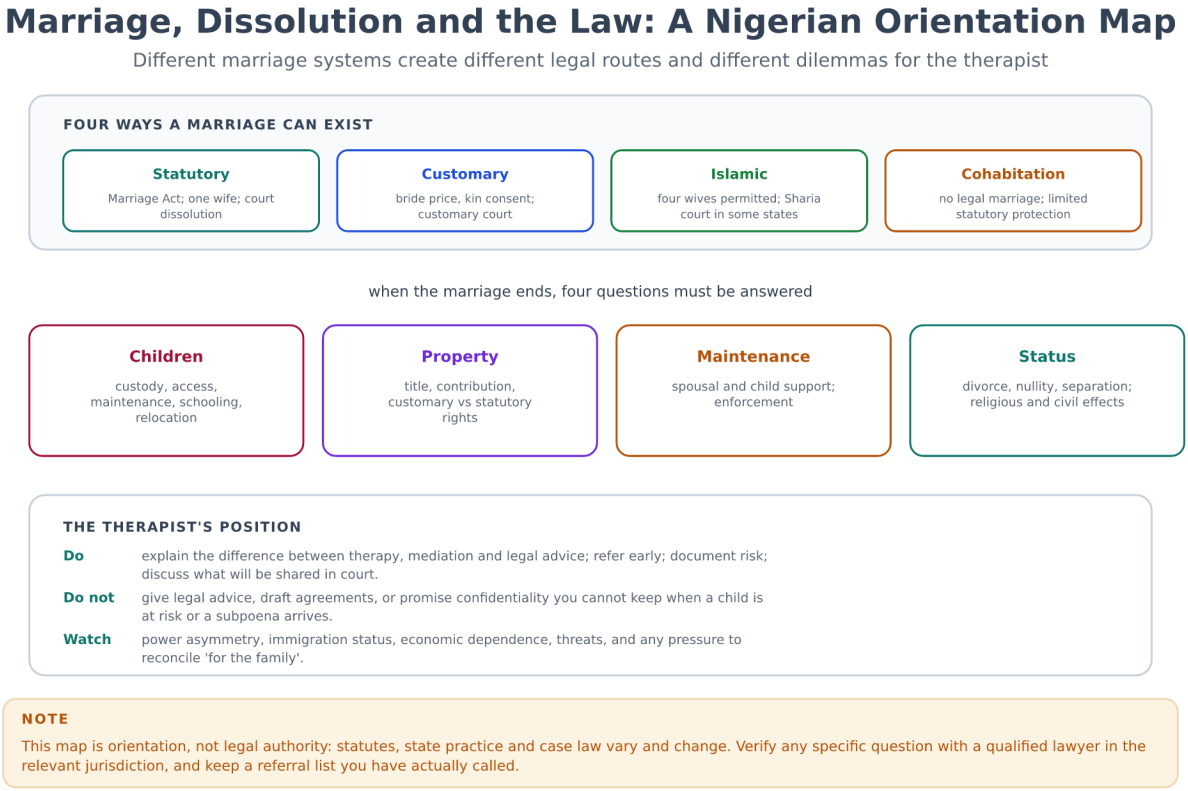


Figure 1.1 Marriage systems, dissolution questions and the therapist's position

Week 2 Four marriage systems

System	Legal basis	Form	Dissolution route
Statutory	Marriage Act	Monogamous; registered	High Court petition under the Matrimonial Causes Act
Customary	Customary law; state customary courts	Bride price, kin consent; potentially polygynous	Customary court; customary arbitration
Islamic	Islamic law; Sharia courts in relevant states	Up to four wives	Sharia court; talaq, khul', or judicial dissolution
Cohabitation	No marriage		No dissolution; limited statutory protection

Clinical implications of the overlap. A couple may be married under two systems; the customary marriage may not be registered; the religious ceremony may have legal consequences; and children's status may depend on which system applies. Do not assume.

Ask: "How was your marriage formalised? Was it registered? Is there a certificate? Which court would you go to if you needed to?" Then refer to a lawyer for the answer's consequences.

Week 3 Divorce and dissolution

Statutory divorce (orientation). Governed by the Matrimonial Causes Act: a petition to the High Court; a two-year minimum duration (with exceptions); a recognised ground (adultery, unreasonable behaviour, desertion, separation, or living apart); and provision for custody, maintenance, property and costs.

Customary and Islamic dissolution. Different processes, different courts, different evidentiary rules, different outcomes for women in practice. Know where to refer.

The therapist's role.

- Help the couple/clients clarify what they are deciding (discernment).
- Explain that therapy is not a legal process and does not produce evidence for court.
- Support the emotional work alongside the legal process.
- Refer for legal advice early, not at the point of crisis.
- Warn clients that therapy records may be requested in proceedings.

Week 4 Custody, access and the welfare principle

The welfare principle. Decisions about children are governed by the child's best interests, not the parents' preferences or the mother's automatic entitlement.

What courts consider (broadly). The child's wishes (weighted by age and understanding); continuity of care; the capacity of each parent; the child's relationship with each parent; siblings; education and health; and any risk of harm.

The therapist's contribution. You can describe the child's relationships, needs and views (carefully, and with appropriate methods); you can support the parents to reduce conflict; and you can help them construct a parenting plan.

What you should not do. Offer a custody recommendation for your own client. That is an expert assessment requiring independence, a defined methodology and often a court appointment. Role conflict invalidates the opinion and harms the family.

The parenting plan. Residence, contact, holidays, schooling, health decisions, communication, transitions, handovers, new partners, and a dispute-resolution clause. Concrete plans reduce conflict.

Week 5 Maintenance and financial provision

Child maintenance. Both parents have an obligation to maintain their children, under statutory and customary law. Amounts are determined by need, capacity and standard of living; enforcement is a practical problem.

Spousal maintenance. Available in some statutory proceedings; less so in customary and religious frameworks and highly variable in practice.

The therapist's role. Money is clinical material: it carries meaning about fairness, power and abandonment. Help the couple separate the amount from the meaning; encourage full financial disclosure; and refer for legal advice on entitlements.

Practical support. Referral to legal aid, mediation services, and women's rights organisations where appropriate.

Week 6 Child rights and protection

The Child Rights Act (2003) adopted in many but not all Nigerian states sets out the rights of the child, including protection from abuse and neglect, the right to be heard, and provisions on child marriage, child labour, and alternative care. State child protection laws and structures operationalise it.

Your duties.

1. Recognise signs of abuse and neglect.
2. Respond without leading or investigating.
3. Record verbatim.
4. Report through the designated route (state ministry responsible for social welfare; police family unit; designated safeguarding lead).
5. Document the advice received and the action taken.
6. Support the child through the process.

What not to do. Investigate yourself; promise secrecy; confront the alleged abuser; or delay the report while you "gather more information".

Week 7 Domestic violence law

The Violence Against Persons (Prohibition) Act 2015 applies in the FCT and in states that have adopted it; other states have their own laws or rely on criminal provisions. Protection orders, duties of the police and support for victims vary substantially by jurisdiction.

The clinician's knowledge. Where the nearest police family unit is; what a protection order requires; which shelters and legal aid organisations exist; and what happens procedurally to a woman who reports.

The clinician's limits. You are not the police, the lawyer, or the judge. Your role is safety planning, documentation, referral and support and not making the client's decisions for her.

Week 8 Mediation and alternative dispute resolution

What mediation is. A structured, voluntary process in which a neutral third party helps the parties reach their own agreement. It is not therapy and not arbitration.

When it is suitable. Both parties voluntary and safe; no coercive control; reasonably balanced power; an issue that can be negotiated; and willingness to disclose information.

When it is not. Violence or coercion; a severe power imbalance (including literacy, language or economic dependency); untreated substance use or mental illness that prevents participation; and safeguarding concerns.

Structure of a mediation session. Opening and ground rules each party's account identification of issues exploration of interests generation of options negotiation agreement documented review.

The therapist's temptation. To slip into therapy mid-mediation (exploring feelings) or into advice. Explain the frame at the start and hold it.

Week 9 Reports and court

The report structure.

1. Your role, qualifications and the basis of your opinion.
2. Who you saw, when, and with whose consent.
3. Methodology (interviews, observation, measures).
4. Findings, described behaviourally.
5. Opinion, clearly separated and limited to your expertise.
6. Limitations, explicitly stated.
7. Recommendations, if requested and within competence.

Language. "It is my opinion that " not "the court should". No adjectives about character. No speculation about events you did not observe.

Testimony. Answer the question asked; do not volunteer; distinguish fact from opinion from hypothesis; say "I don't know" when you don't; and remain courteous under challenge.

Records. Assume that anything you write may be read in court. Write accordingly.

Week 10 Expert versus treating roles

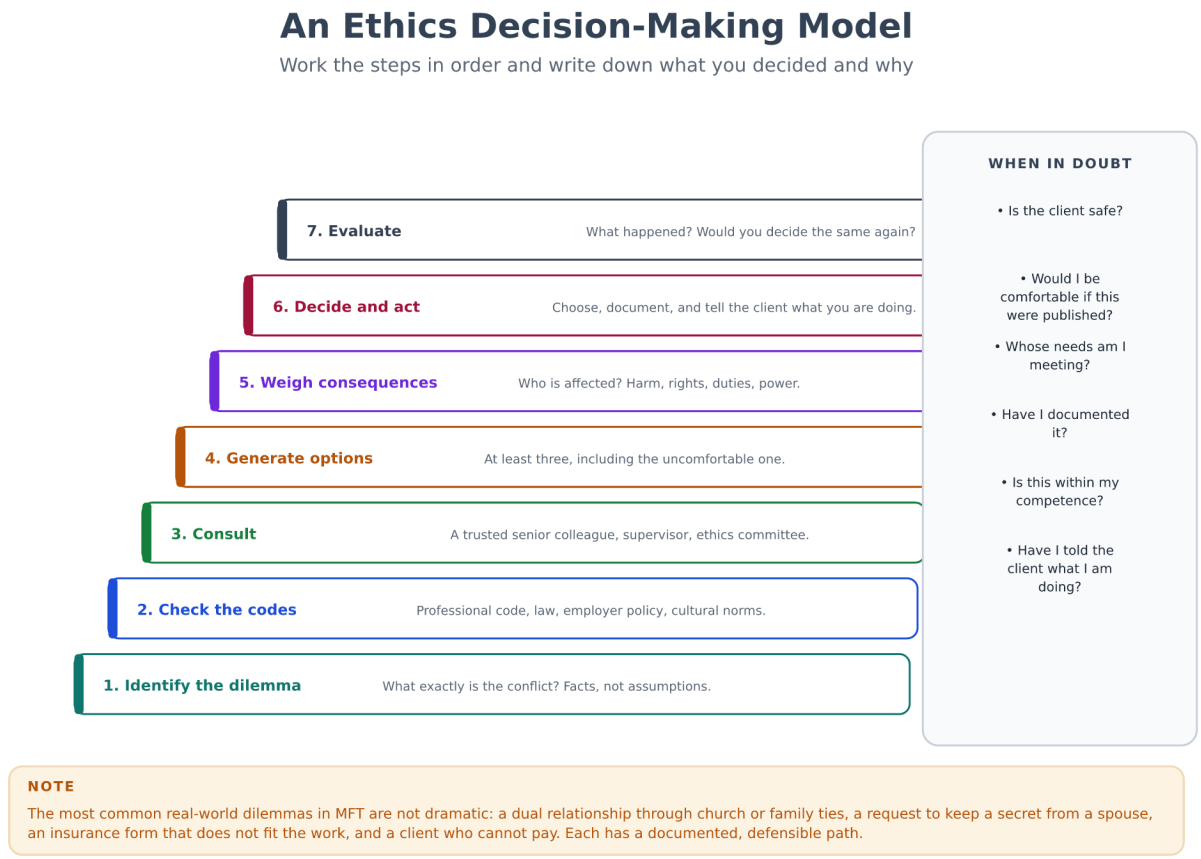


Figure 10.1 An ethics decision model for role conflicts

Treating clinician	Expert assessor
Paid by/for the client; therapeutic duty	Paid by the court; duty to the court
Confidential	Limited confidentiality; report disclosed
Advocates for the client's wellbeing	Neutral; reports what the evidence shows
Continuous relationship	Time-limited, defined assessment
Cannot be objective about custody	Can be objective if independent

The conflict is structural, not a matter of willpower. Accepting both roles compromises the therapy and the evidence. If asked, explain and refer.

Where you can help. Provide factual records with consent and a court order as required; support the client through the process; and help parents reduce conflict.

Week 11 Working with lawyers and courts

What lawyers need from you. Clear, dated, factual information; a timely response; an opinion you can defend; and an understanding of your limits.

What you need from them. Clarity about the purpose of the report; the legal questions to address; the deadline and format; and confirmation of consent arrangements.

Interprofessional etiquette. Do not tell a lawyer their job; do not let a lawyer tell you your clinical findings. Negotiate the boundary explicitly and in writing.

Week 12 Mediation skills assessment

Thirty minutes, role-played. Assessed on: framing and ground rules; neutrality; screening for safety; interest-based exploration; option generation; documentation; and referral decisions.

Week 13 Advocacy

From case to policy. Individual work reveals systemic failures: no shelter, no legal aid, no interpreter, no protection order enforcement, no follow-up. Advocacy is the professional response.

Levels.

1. **Case advocacy** with one client: writing a letter, making a call, accompanying to a service (with care).
2. **Service advocacy** improving your own organisation's pathway.
3. **Policy advocacy** submissions, research briefs, media, coalitions.
4. **Community advocacy** working with community organisations and faith groups.

The evidence base for advocacy. Documented case patterns, service data, and research. Anecdote persuades sympathetic listeners; data persuades sceptical ones.

Ethical guardrails. Consent before using a client's story; never identify a client; separate your professional opinion from your personal politics; and do not compromise clinical relationships for a campaign.

Week 14 Advocacy proposal and integration

Present a 1,500-word proposal: the problem, the evidence, the target, the ask, the allies, the timeline, and how you will know whether it worked. Then, in the final discussion, return to the boundary map from Week 1 and revise it with what you have learned.

Glossary

ADR alternative dispute resolution. **Coercive control** a pattern of domination and isolation that makes mediation unsafe. **Guardian ad litem** a person appointed to represent a child's interests in proceedings. **Maintenance** financial support owed to a spouse or child. **Matrimonial Causes Act** the Nigerian statute governing statutory divorce. **Protection order** a court order restraining a person from specified conduct. **Welfare principle** the rule that the child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. A therapist asked by a client "will I get custody?" should: (a) predict the outcome (b) explain the welfare principle and refer for legal advice (c) refuse to speak (d) advise on strategy

2. Conducting a custody evaluation of your own client is: (a) acceptable with consent (b) a role conflict (c) required (d) ideal

3. Mediation is contraindicated when: (a) parties disagree (b) there is coercive control (c) there are children (d) money is involved

4. The welfare principle concerns: (a) the parents' rights (b) the child's best interests (c) the court's convenience (d) the family's income

5. A child protection concern should first be: (a) investigated by you (b) recorded and reported through the designated route (c) discussed with the family (d) held until you are certain

6. In mediation the therapist should: (a) explore childhood (b) hold the mediation frame (c) give legal advice (d) take sides

7. An expert report should separate: (a) facts from opinions (b) parents from children (c) fees from findings (d) nothing

8. Advocacy with a client's story requires: (a) no consent (b) informed consent and anonymisation (c) media involvement (d) the client's name

9. The four marriage systems include all except: (a) statutory (b) customary (c) Islamic (d) corporate

10. Therapy records in litigation should be treated as: (a) permanently confidential (b) potentially disclosable (c) irrelevant (d) destroyed immediately

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 a 8 b 9 d 10 b

Short answer

1. Distinguish therapy, mediation, expert assessment and legal advice.
2. What are the grounds for refusing to mediate?
3. Outline a court-facing report structure.
4. List the steps when you suspect child abuse.
5. Give three examples of legitimate professional advocacy.

Essay (choose one, 2,000 words)

1. "The therapist's duty is to the family, the expert's duty is to the court." Analyse the consequences.

2. Critically examine the protection available to victims of domestic violence in your jurisdiction.
3. Design an advocacy campaign to improve one aspect of family justice in your context.

Further reading

- Matrimonial Causes Act (Nigeria), Cap M7 LFN 2004.
- Child Rights Act (Nigeria), 2003.
- Violence Against Persons (Prohibition) Act, 2015.
- Stahl, P. M. (2011). Conducting Child Custody Evaluations (2nd ed.). Sage.
- Emery, R. E. (2012). Renegotiating Family Relationships (2nd ed.). Guilford.
- Barsky, A. E. (2016). Clinicians in Court (2nd ed.). Guilford.

Next: MFT 725 Integrative Approaches in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 26 OF 28

MFT 725

Integrative Approaches in Therapy

14 teaching weeks 1 figure

ELECTIVES

1.71 units Prerequisite: MFT 702, 715

> Most experienced clinicians are integrative, whether or not they say so.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0

MFT 725 Integrative Approaches in Therapy

MSc Marriage and Family Therapy Elective 1.71 Units 14 teaching weeks Prerequisite: MFT 702, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 725 Integrative Approaches in Therapy. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	**Integrative Approaches in Therapy**	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 Why integrate . . . 6

Week 2 Forms of integration . . . 7

Week 3 Common factors . . . 8

Week 4 The systemic dimension . . . 8

Week 5 The attachment dimension . . . 8

Week 6 The behavioural and cognitive dimensions . . . 9

Week 7 The experiential and narrative dimensions . . . 9

Week 8 The biological dimension . . . 9

Week 9 Matching, sequencing and staging . . . 9

Week 10 Feedback-informed treatment . . . 10

Week 11 Comorbidity and complexity . . . 10

Week 12 Culture, values and the ethics of integration . . . 11

Week 13 Coherence . . . 11

Week 14 Your integrative framework . . . 11

Glossary . . . 12

Self-assessment . . . 12

- Multiple choice . . . 12
- Answer key . . . 12
- Short answer . . . 12
- Essay (choose one, 2,000 words) . . . 13

Further reading . . . 13

Course leader's summary. Most experienced clinicians are integrative, whether or not they say so. The question is whether the integration is disciplined or accidental. This course builds a principled framework for combining models without producing incoherence, and teaches you to justify every combination.

How to use this book

Integration is a design problem, not a mood. Each week adds a dimension to a framework you will use for the rest of your career. By Week 14 you will have a written integrative framework, tested on three cases, and defended in a viva.

1. Course description

The course covers: the varieties of integration (assimilative, theoretical, sequential, pluralistic); common factors and their implications; the major dimensions of family therapy (systemic, attachment, behavioural, experiential, cognitive, narrative, biological); decision frameworks for matching model to client and phase; feedback-informed treatment; staged and stepped care; and the risks of eclecticism.

2. Learning outcomes

#	Outcome	Assessed
LO1	Distinguish the forms of integration and their assumptions	Exam
LO2	Explain common factors and use them to justify technique selection	Exam
LO3	Construct a coherent integrative formulation for a complex case	Case study
LO4	Sequence models across phases with an explicit rationale	Treatment plan
LO5	Use routine outcome monitoring and feedback to guide integration	Portfolio
LO6	Avoid incoherence and demonstrate coherence to a supervisor	Viva
LO7	Articulate a personal integrative framework	Framework paper

3. Assessment

Component	Weight	Due
Integrative case study (2,500 words)	35%	Week 8
Treatment plan with sequenced models (1,500 words)	25%	Week 11
Outcome monitoring portfolio	15%	Week 13
Framework paper and viva (2,000 words)	25%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why integrate: evidence, clients, and clinicians	Integration audit of your own practice
2	Forms of integration	Mapping four forms onto a case
3	Common factors and their clinical use	Common-factors observation
4	The systemic dimension	Systemic formulation
5	The attachment dimension	Attachment-informed plan
6	The behavioural and cognitive dimensions	Skills module design
7	The experiential and narrative dimensions	Emotion and meaning work
8	**Case study due**; the biological dimension	Medication-aware plan
9	Matching, sequencing and staging	Sequence design
10	Outcome monitoring and feedback-informed treatment	ORS/SRS practice
11	**Treatment plan due**; comorbidity and complexity	Complex case lab
12	Culture, values and the ethics of integration	Values audit
13	**Portfolio due**; coherence and supervision	Coherence defence
14	**Framework paper and viva**	Assessed

Week 1 Why integrate

Three pressures.

1. **Evidence.** No single model is superior for everything. Different models have stronger evidence for different problems (FBT for adolescent anorexia, EFT for couple distress, PMT for conduct problems, family psychoeducation for psychosis).
2. **Clients.** Real families present with multiple, interacting problems: trauma plus addiction plus couple distress plus a child's school refusal.
3. **Clinicians.** Most experienced therapists drift towards integration as their caseload diversifies.

The danger. Integration without discipline becomes eclecticism: a technique for every mood, no coherent theory of change, and a client who cannot tell what is expected of them.

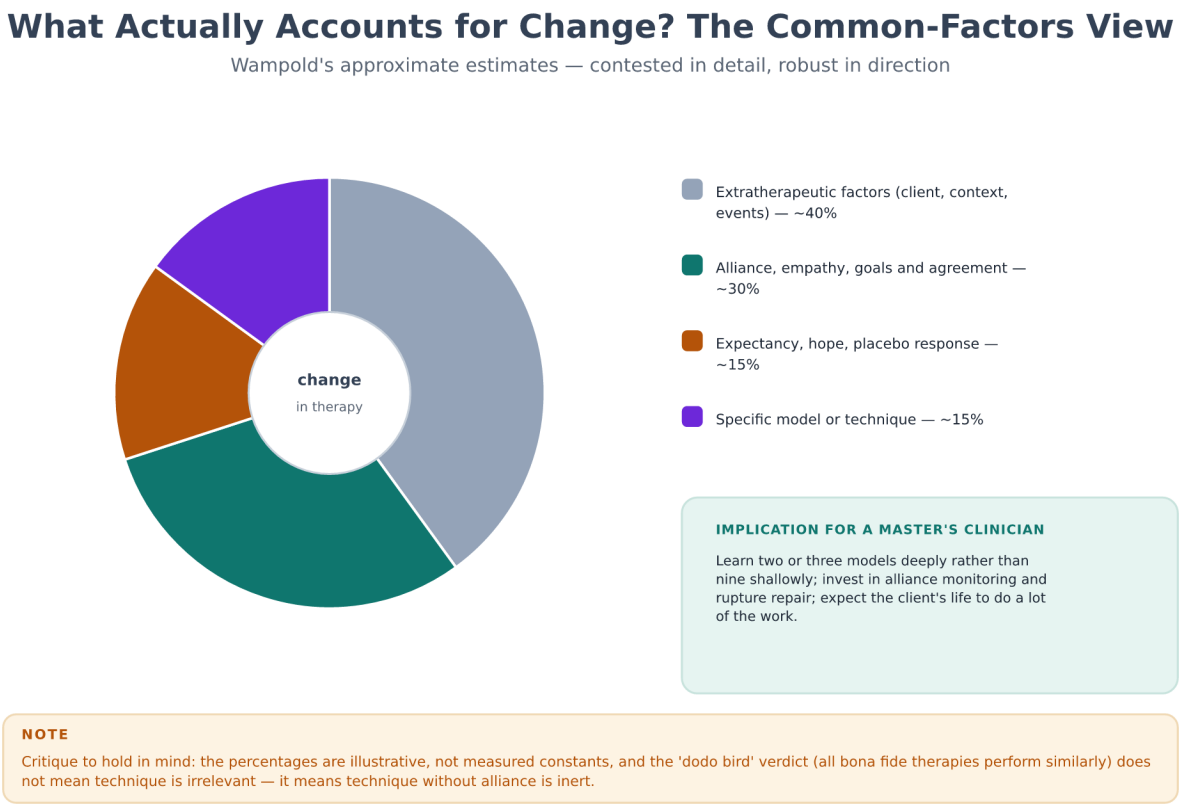


Figure 1.1 What accounts for change: a common-factors view

AUDIT

List the models you actually use. For each, name the client problem you use it for and the theory of change you rely on. Identify any technique you use without being able to explain why.

Week 2 Forms of integration

Form	Definition	Example	Risk
Assimilative	A home model, enriched by techniques from elsewhere	EFT therapist borrowing a behavioural task	Drift from the core theory
Theoretical	A new model synthesising several	Integrative behavioural couples therapy	Complexity, training burden
Sequential	Different models at different phases	Structural stabilisation, then EFT	Discontinuity for the family
Parallel	Different models for different subsystems	Couple work + individual trauma work	Splitting between clinicians
Pluralistic	Collaborating with the client on method	"Which approach fits you?"	Requires an informed client

Choosing a form. Assimilative integration is the most realistic for a newly qualified clinician: it keeps coherence while allowing flexibility. Sequential and parallel integration are appropriate for complexity. Pluralistic practice is an ethical stance as much as a technique.

Week 3 Common factors

The claim. Much of psychotherapy's effect comes from factors shared across models: the alliance, agreement on goals, therapist empathy, client expectancy, and extratherapeutic factors.

The clinical use without relativism.

1. **Monitor the alliance** in every session, with every family member.
2. **Repair ruptures** quickly; the repair is often more powerful than the original intervention.
3. **Agree goals explicitly**; disagreement about goals predicts drop-out.
4. **Engender hope** honestly (not false reassurance).
5. **Then use techniques** which matter most for specific problems and for structuring the work.

The critique. Percentages attributed to common factors are illustrative, not measured; "all models are equal" is over-stated; and technique does matter for specific disorders. The defensible position: the alliance is necessary but not sufficient; technique without alliance is inert; alliance without technique is often insufficient.

Week 4 The systemic dimension

What it contributes. Attention to pattern, structure, triangles, context and power. It tells you where to look and what maintains the problem.

What it lacks alone. Specific techniques for symptom change; guidance for individual emotion and trauma; and a sufficient account of biology.

Integration in practice. Systemic formulation + any intervention. You can use a behavioural task, an EFT enactment, or a narrative externalising conversation, provided the systemic formulation justifies it.

Week 5 The attachment dimension

What it contributes. A developmental account of why bonds matter; a map of strategies under threat; a mechanism for change (corrective emotional experience, enacted in the room).

Integration with systemic work. Systemic thinking tells you the loop; attachment tells you why the loop matters so much and what each person is asking for.

Integration with behavioural work. Behavioural tasks build positive exchange; attachment work deepens the meaning of the exchange. Alone, either can be shallow.

Week 6 The behavioural and cognitive dimensions

What they contribute. Specific, testable, teachable skills; measurable change; homework; structure; and a strong evidence base for many problems.

Integration cautions. Behavioural work can feel mechanical when the emotional bond is damaged; cognitive restructuring can be premature in acute trauma; and skills practice without meaning can become performance.

A practical sequence. Stabilise and structure (behavioural) access emotion and meaning (experiential, attachment) consolidate with skills and plans (behavioural, cognitive).

Week 7 The experiential and narrative dimensions

What they contribute. Access to primary emotion; meaning-making; identity; and the language of the family's own story.

Integration. Experiential work deepens the emotional field; narrative work organises it into a story the family can hold. Together they answer "what do we feel?" and "what does it mean?".

Caution. Emotional work without containment can flood; narrative work without emotion can become intellectual. The integration is in the pacing.

Week 8 The biological dimension

What it contributes. Sleep, medication, substances, hormonal change, illness, and the physiology of stress and trauma.

Integration. A formulation that ignores biology will fail in predictable ways: the depressed client who does not respond to therapy because of untreated hypothyroidism; the couple whose sexual problem is a medication side effect; the adolescent whose sleep deprivation mimics ADHD.

Your role. Screen, refer, coordinate, and never advise on medication.

Week 9 Matching, sequencing and staging

Decision framework five questions.

1. **What is the most dangerous thing here?** (Risk governs sequence.)
2. **What is the most changeable thing here?** (Start where change is possible; hope is built on early wins.)

3. **What must be in place before other work can happen?** (Safety, stabilisation, attendance, sobriety.)
4. **Which model has the best evidence for the primary problem?** (Use it, or refer.)
5. **What does this family want and believe in?** (Engagement is a precondition, not a luxury.)

Staging example. A couple with active violence, alcohol misuse, and a child's school refusal: Phase 1 safety and sobriety (individual, specialist referral); Phase 2 couple de-escalation and parenting plan; Phase 3 child's return to school and family consolidation.

Week 10 Feedback-informed treatment

The instruments. The Outcome Rating Scale (ORS) and Session Rating Scale (SRS) are brief, free, and practical: four visual analogue items each, completed every session.

How to use them.

1. Introduce them as a way of making sure the therapy is working.
2. Look at the scores before you speak.
3. Ask about a drop: "Your score fell by three points. What happened?"
4. Ask about the alliance: "Is there anything about how we work that is not fitting?"
5. Act on the answer in the session, not next month.

The evidence. Routine outcome monitoring with feedback improves outcomes and reduces deterioration, particularly for clients who are not progressing. It is one of the most robust findings in the field.

> **PORTFOLIO (15%)**
> Ten sessions with ORS/SRS data, your interpretation, and the changes you made.

Week 11 Comorbidity and complexity

Complexity is the norm. Typical community caseloads include trauma plus substance use plus couple distress plus child behaviour plus financial strain.

Principles.

1. Formulate the whole system, not a list of diagnoses.
2. Identify the keystone: the one change that would make others possible.
3. Sequence: safety stabilisation skills processing consolidation.
4. Share the load: refer where specialist input is needed; coordinate rather than compete.
5. Review: complexity requires more frequent review, not longer sessions.

Week 12 Culture, values and the ethics of integration

Culture shapes which models fit. An approach that prizes emotional expressiveness may feel exposing; one that prizes autonomy may feel cold; one that requires homework may be impractical.

Values shape technique choice. Your beliefs about gender, family duty, forgiveness and individual autonomy will show up in what you emphasise. Audit them.

Ethical integration. Do not use a technique you are not trained in; do not combine models to appear sophisticated; and do not use integration as a reason to avoid referring.

Week 13 Coherence

The coherence test.

Question	If the answer is no
Can I state the formulation in three sentences?	You do not have one yet
Does every intervention follow from the formulation?	You are improvising
Can I name the model I am using at this moment?	You are drifting
Can I predict what will happen next, and be wrong productively?	Your formulation is too vague
Would a supervisor understand my reasoning without me explaining it?	Document it

The coherence defence. Present a case to peers and answer: what are you doing, why, and how will you know whether it worked?

> **COHERENCE DEFENCE**
> Ten minutes with a case, followed by hostile questioning from peers.

Week 14 Your integrative framework

Write 2,000 words:

1. Your home model and its theory of change.
2. The dimensions you integrate (systemic, attachment, behavioural, experiential, narrative, biological).
3. Your decision rules for matching and sequencing.
4. Your use of feedback and outcome data.
5. Your limits: what you do not do, and to whom you refer.
6. How you will know you are drifting.

Defend it in a viva.

Glossary

Assimilative integration a home model enriched by techniques from elsewhere. **Common factors** mechanisms shared across therapies (alliance, empathy, expectancy, goals). **Feedback-informed treatment** using session-by-session outcome and alliance data to adjust therapy. **Keystone change** a change that makes other changes possible. **Pluralistic practice** collaborating with the client to choose methods. **Sequential integration** using different models at different phases of treatment. **Stepped care** matching intensity of intervention to need.

Self-assessment

Multiple choice

1. The main risk of undisciplined integration is: (a) cost (b) incoherence for the family (c) slow progress (d) paperwork

2. Assimilative integration means: (a) equal blending (b) a home model enriched by borrowed techniques (c) alternating models by session (d) refusing a model

3. Common factors research suggests: (a) technique is irrelevant (b) alliance matters greatly and technique matters for specific problems (c) all models are identical (d) diagnosis is useless

4. The first question in sequencing is: (a) what is most interesting (b) what is most dangerous (c) what is cheapest (d) what the therapist prefers

5. Feedback-informed treatment uses: (a) long questionnaires (b) brief session-by-session measures such as ORS/SRS (c) only diagnosis (d) supervisor ratings

6. A keystone change is: (a) the first session (b) a change that makes others possible (c) a diagnosis (d) a referral

7. The dimensional approach includes all except: (a) systemic (b) attachment (c) behavioural (d) astrological

8. Integration is unethical when it involves: (a) borrowing a technique (b) using techniques outside your competence (c) sequencing models (d) monitoring outcomes

9. Coherence is demonstrated by: (a) using many techniques (b) a formulation from which interventions follow (c) longer sessions (d) confidence

10. Complexity should lead to: (a) longer sessions (b) more frequent review and coordination (c) more diagnoses (d) abandoning formulation

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 d 8 b 9 b 10 b

Short answer

1. Distinguish four forms of integration with an example of each.

2. Give three clinical uses of common-factors research.
3. State the five sequencing questions.
4. How do you use ORS/SRS data in a session?
5. What are three ethical limits on integration?

Essay (choose one, 2,000 words)

1. "Integration without a theory of change is improvisation." Discuss with a case.
2. Design an integrative treatment plan for a family with trauma, addiction and couple distress.
3. Critically evaluate feedback-informed treatment as a professional standard.

Further reading

- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Duncan, B. L., Miller, S. D., & Sparks, J. A. (2004). The Heroic Client. Jossey-Bass.
- Norcross, J. C., & Goldfried, M. R. (Eds.). (2019). Handbook of Psychotherapy Integration (3rd ed.). OUP.

Next: MFT 726 Neurobiology of Relationships

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 27 OF 28

MFT 726

Neurobiology of Relationships

14 teaching weeks 4 figures

ELECTIVES

1.71 units Prerequisite: MFT 714

> Neuroscience does not replace systemic thinking; it supplies the mechanism.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0

MFT 726 Neurobiology of Relationships

MSc Marriage and Family Therapy Elective 1.71 Units 14 teaching weeks Prerequisite: MFT 714

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 726 Neurobiology of Relationships. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	**Neurobiology of Relationships**	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 Why physiology belongs in family therapy . . . 6

Week 2 The autonomic nervous system . . . 7

Week 3 Co-regulation . . . 7

Week 4 Polyvagal theory: use and critique . . . 7

Week 5 Stress and the HPA axis . . . 9

Week 6 Attachment as biology . . . 10

Week 7 The social brain . . . 11

Week 8 Teaching regulation . . . 12

Week 9 Memory, trauma and the body . . . 12

Week 10 Couple conflict physiology . . . 13

Week 11 Sleep, exercise, substances and mood . . . 13

Week 12 Adolescent and ageing brains . . . 13

Week 13 Neuroplasticity and its limits . . . 14

Week 14 Integration . . . 14

Glossary . . . 14

Self-assessment . . . 14

- Multiple choice . . . 14
- Answer key . . . 15
- Short answer . . . 15
- Essay (choose one, 2,000 words) . . . 15

Further reading . . . 15

Course leader's summary. Neuroscience does not replace systemic thinking; it supplies the mechanism. This course teaches what is well established, what is contested, and what is oversold so that you can use physiology to help families regulate, without pretending to be a neuroscientist.

How to use this book

The field is full of confident claims that outrun the evidence. Each week distinguishes three levels: **robust** (well replicated), **plausible** (reasonable but uncertain), and **popular** (widely repeated, weakly supported). You are expected to be able to say which is which.

1. Course description

The course covers the autonomic nervous system and co-regulation; the social brain; stress physiology and the HPA axis; attachment as a biological system; memory and trauma; the physiology of couple conflict; sleep, exercise and substances; adolescent and ageing brains; neuroplasticity; and the responsible use of neuroscience in clinical communication.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the autonomic, endocrine and central mechanisms relevant to relationships	Exam
LO2	Use physiological knowledge to pace and structure sessions	Observed session
LO3	Teach regulation skills accurately and without over-claiming	Skills test
LO4	Explain the physiology of couple conflict and repair	Case study
LO5	Distinguish robust from popular claims in the literature	Critical appraisal
LO6	Integrate physiological and systemic formulations	Case study
LO7	Communicate neuroscience to families in accessible language	Viva

3. Assessment

Component	Weight	Due
Critical appraisal of a "neuro" claim (1,500 words)	20%	Week 5
Regulation skills teaching (20 min)	25%	Week 8
Integrative case study (2,500 words)	35%	Week 12
Examination	20%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why physiology belongs in family therapy	Physiology in a case
2	The autonomic nervous system	Vagal braking demonstration
3	Co-regulation and the dyad	Co-regulation observation
4	Polyvagal theory: use and critique	Ladder mapping
5	**Appraisal due**; stress and the HPA axis	Stress mapping
6	Attachment as biology	Attachment formulation
7	The social brain	Neuroanatomy for clinicians
8	**Regulation skills teaching**	Assessed
9	Memory, trauma and the body	Trauma-informed pacing
10	Couple conflict physiology	Flooding and repair
11	Sleep, exercise, substances and mood	Lifestyle screening
12	**Case study due**; adolescent and ageing brains	Developmental adaptation
13	Neuroplasticity and the limits of the claim	Critical reading
14	Integration and examination	Review

Week 1 Why physiology belongs in family therapy

Four clinical facts that physiology explains.

1. Why a person cannot "just calm down" mid-argument (autonomic arousal; prefrontal resources offline).
2. Why a parent's calm voice regulates a child (co-regulation before self-regulation).
3. Why the same conversation is possible at 10 a.m. and impossible at 11 p.m. (fatigue, cortisol, blood sugar).
4. Why trauma changes memory and reaction (implicit memory, sensitised threat detection).

What physiology does not explain. Meaning, culture, power, loyalty and choice. A physiological account is a mechanism, not a formulation. It tells you how the system works; it does not tell you what the family's behaviour means.

The rule of use. Use physiology to (a) pace the session, (b) teach regulation, (c) reduce shame ("your body is doing its job"), and (d) justify referral. Do not use it to explain away relational responsibility.

Week 2 The autonomic nervous system

Two branches, three states (simplified).

- **Sympathetic** mobilisation: heart rate up, pupils dilate, digestion down, muscles tense. Fight or flight.
- **Parasympathetic** rest and digestion, plus (via the ventral vagal pathway) social engagement.
- **Dorsal vagal shutdown** immobilisation: numbness, collapse, dissociation.

Robust findings. The autonomic system responds rapidly and largely outside awareness; arousal affects cognition; chronic stress alters baseline functioning; breathing and movement can shift arousal.

Plausible but contested. The precise mapping of "ventral vagal" states onto social behaviour is a theoretical proposal (Porges) with partial empirical support; the popular version often overstates what is known.

Clinical translation. Teach simple, reliable regulation: slower exhale, orienting, movement, grounding, cool water. These work regardless of which theory explains them.

Week 3 Co-regulation

The concept. Regulation develops in relationships: a calm adult's voice, face, breathing and touch regulate an infant's arousal before the infant can regulate alone.

Robust. Infants synchronise with caregivers on multiple channels; caregiver depression and unresponsiveness affect infant physiology and behaviour; parental calm reduces child distress in experimental settings.

Clinical use.

1. In session, your own regulated state is an intervention: slow your speech, lower your volume, breathe out slowly.
2. Teach parents to co-regulate before reasoning: "First calm the body, then talk about the behaviour."
3. In couple work, teach partners to be a regulating presence rather than a stimulus.

The asymmetry to remember. Co-regulation is not always available. A dysregulated parent cannot lend a calm nervous system. Support the adult first.

Week 4 Polyvagal theory: use and critique

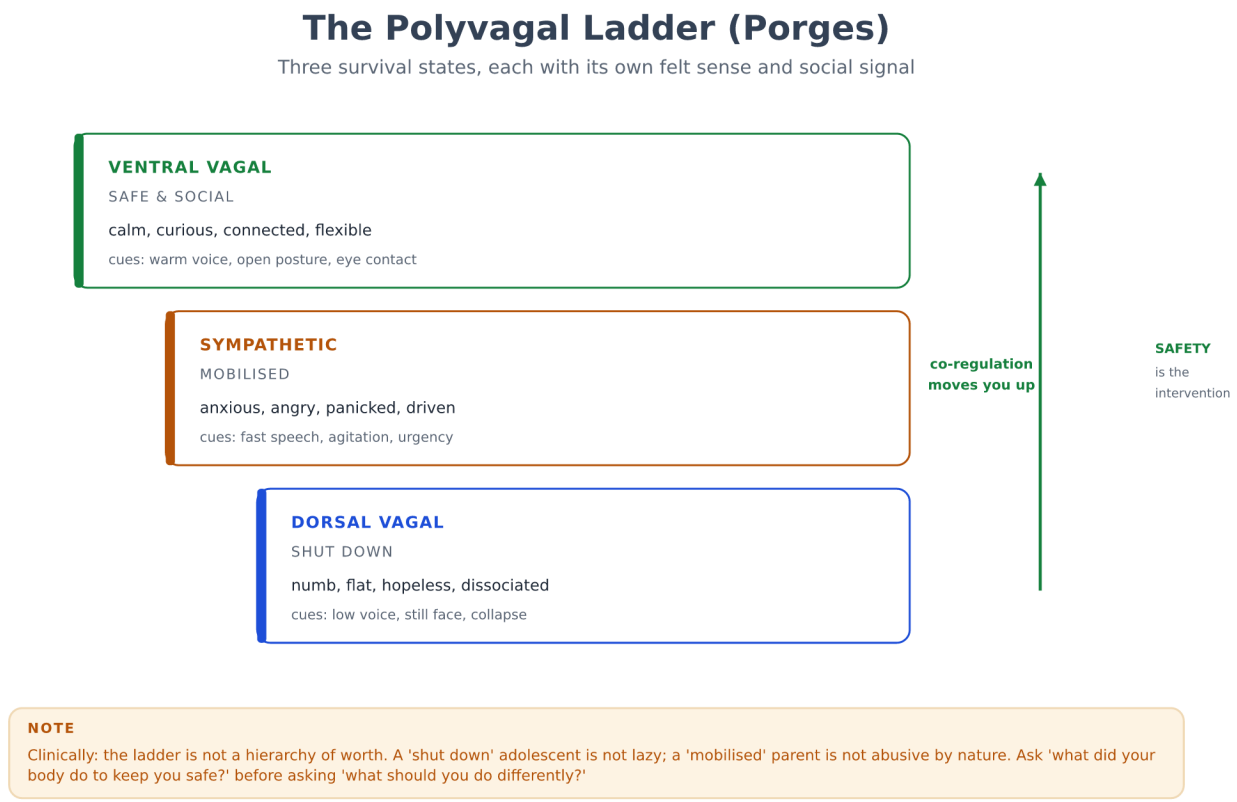


Figure 4.1 The polyvagal ladder

What the theory claims. Three phylogenetically ordered states ventral vagal (safe/social), sympathetic (mobilised), dorsal vagal (shutdown) with the ventral vagal system enabling social engagement via myelinated pathways.

What is useful clinically. The ladder metaphor helps clients recognise state; "neuroception" (unconscious threat detection) explains reactions that seem disproportionate; and the emphasis on safety and co-regulation is good practice.

What is contested. The evolutionary claims, the strict correspondence between anatomy and behaviour, and the tendency to treat the three states as discrete and universal. Critics note the evidence is thinner than the popularity suggests.

The professional stance. Use the metaphor, label it as a metaphor, and do not present contested claims as established fact. Clients benefit from the map, not from a lecture on vagal anatomy.

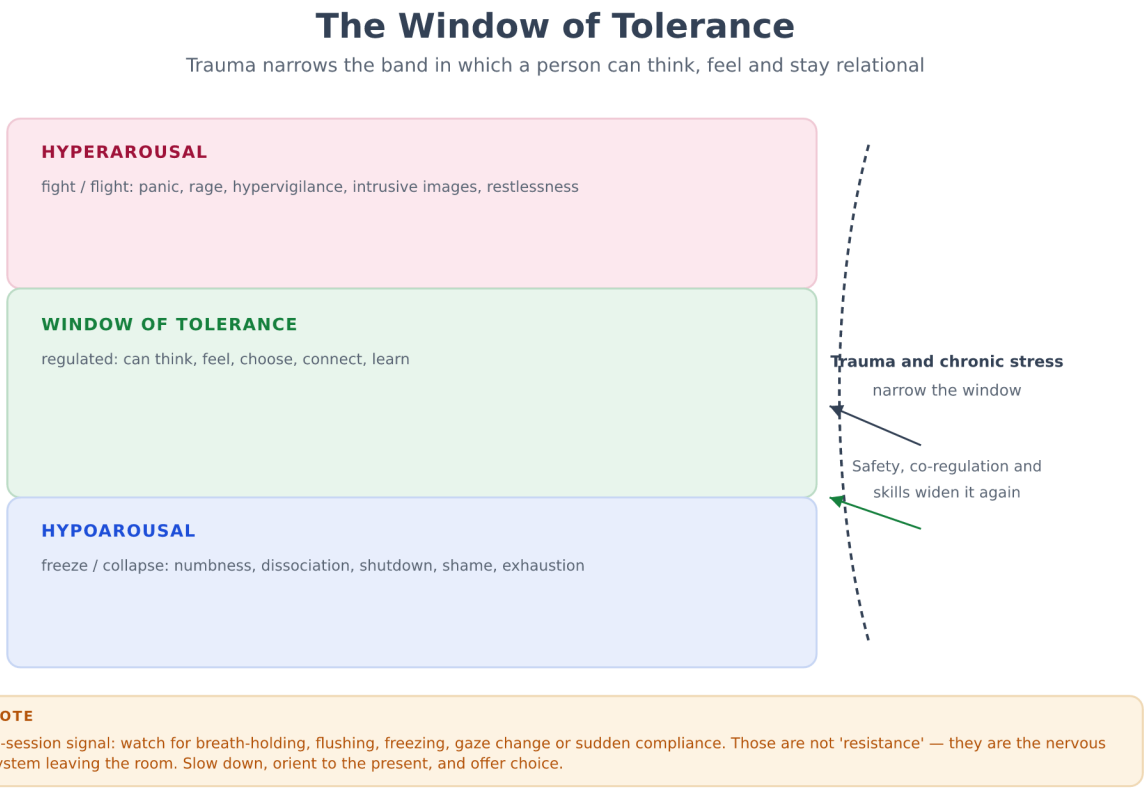


Figure 4.2 The window of tolerance

Week 5 Stress and the HPA axis

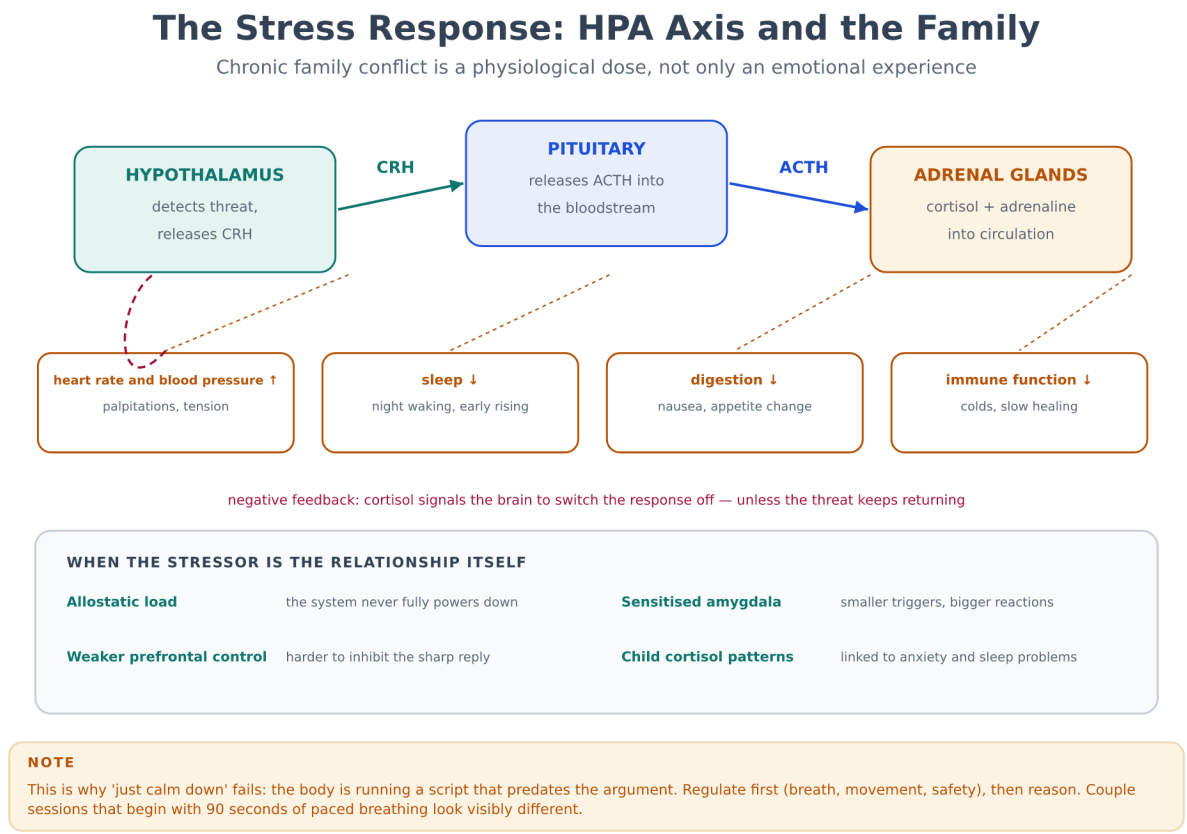


Figure 5.1 The HPA axis and chronic family stress

The pathway. Hypothalamus (CRH) pituitary (ACTH) adrenal glands (cortisol) feedback to the brain. Acute stress is adaptive; chronic stress produces allostatic load.

Robust findings. Chronic stress is associated with altered cortisol patterns, impaired immune function, sleep disruption, cardiovascular risk, and (in children) altered stress reactivity.

Plausible. Chronic relational conflict functions as a chronic stressor with measurable physiological effects. The size of the effect, and the direction of causality, remain debated.

Clinical translation.

- Screen for sleep, exercise, caffeine, alcohol, and medical conditions.
- Teach that physiological state affects the conversation: "You are not bad at communicating; you are tired at 9 p.m."
- Justify referral: persistent fatigue, weight change, palpitations, unexplained pain.

Week 6 Attachment as biology

Robust. Attachment behaviours are observable and cross-culturally recognisable; caregiver responsiveness predicts later social and emotional outcomes; severe early deprivation has lasting effects (Romanian orphan studies).

Plausible. Attachment patterns are encoded as expectations and physiological set-points, and can be revised by later relationships.

Popular but overstated. Claims that specific brain regions "store" attachment, or that a single relationship "rewires" the brain in a defined number of weeks.

Clinical use. The attachment frame explains why a partner's tone matters more than their argument, and why therapy must provide a predictable relational experience before it can provide insight.

Week 7 The social brain

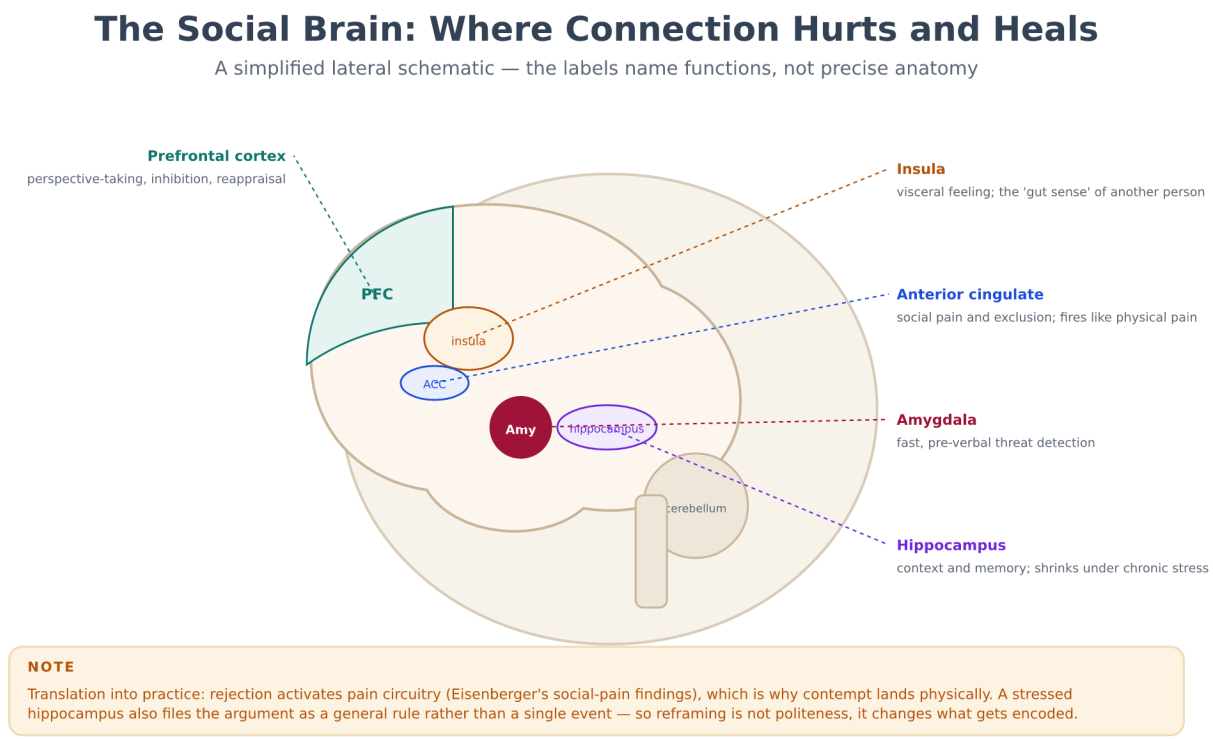


Figure 7.1 A simplified social brain map

Regions and functions (functionally, not precisely).

Region	Function relevant to therapy
Prefrontal cortex	Perspective-taking, inhibition, reappraisal offline under stress
Amygdala	Rapid threat detection, pre-verbal alarm
Insula	Interoception; the "gut sense" of another person
Anterior cingulate	Social pain, exclusion, conflict monitoring
Hippocampus	Context and memory; vulnerable to chronic stress
Mirror system (contested)	Imitation and resonance; overstated in popular accounts

Robust. Social exclusion activates pain-related circuitry (Eisenberger); reappraisal changes emotional response; stress impairs prefrontal function.

Contested. "Mirror neurons" as an explanation of empathy; the "right brain/left brain" attachment story; "the amygdala hijack" as a precise mechanism.

Clinical translation. Rejection hurts physically say so, because it reduces shame. Reappraisal works but requires a regulated brain so regulate first.

Week 8 Teaching regulation

Assessed: teach a regulation skill in 20 minutes so that a family can use it at home.

A teaching sequence.

1. **Explain briefly** why the body matters (30 seconds, no lecture).
2. **Demonstrate** the skill yourself.
3. **Practise together** in the session.
4. **Troubleshoot** ("what would make this hard at home?").
5. **Anchor it** to a specific moment ("when you feel your jaw tighten").
6. **Write it down** and review next session.

Four reliable skills.

Skill	How	When
Extended exhale breathing	In for 4, out for 6 8, for 90 seconds	Before a difficult conversation
Orienting	Slowly look around, name five things	Dissociation, panic
Grounding	Feet, weight, texture, temperature	Trauma intrusion
Movement	Stand, walk, stretch, shake out	After conflict; restlessness

---

Week 9 Memory, trauma and the body

Robust. Traumatic memories are often fragmented and sensory; stress hormones affect memory consolidation; avoidance maintains fear; exposure-based treatments work.

Plausible. Implicit memory contributes to reactions without a narrative; the body "remembers" in the sense that physiological responses are conditioned.

Popular but false. That traumatic memories are stored "in the muscles" and released by massage; that the body has a literal memory independent of the brain; that a single technique can "release" trauma.

Clinical translation.

- Do not require a narrative before stabilisation.
- Expect sensory fragments and treat them as memory, not madness.
- Use grounding to establish "now" versus "then".
- Refer for trauma-specific therapy when indicated; do not improvise.

Week 10 Couple conflict physiology

Robust. Physiological arousal during conflict predicts poor problem-solving; high arousal narrows attention and reduces empathy; recovery after conflict predicts relationship outcomes.

Gottman's flooding threshold (roughly 100 bpm) is a clinical heuristic rather than a precise cut-off, but the principle is sound: above a certain arousal, listening is physiologically difficult.

Clinical use.

1. Teach a **break protocol**: agree a signal, separate for 20 30 minutes, self-regulate, return.
2. Explain that the break is not abandonment; it is a condition for a useful conversation.
3. Do not hold difficult conversations late at night, when tired, hungry, or after alcohol.
4. In session, watch for signs of flooding and pause before continuing.

Week 11 Sleep, exercise, substances and mood

Sleep. Robustly linked to mood, irritability, attention and emotion regulation. Ask about it in every assessment, and treat it as a clinical target, not a lifestyle issue.

Exercise. Modest but reliable effects on mood and anxiety; a useful adjunct.

Alcohol and cannabis. Disrupt sleep architecture, increase impulsivity, and reduce inhibition all relevant to couple conflict and parenting.

Caffeine and screens. Contribute to sleep disruption; worth asking about without moralising.

The clinical frame. "Let us look at what your body is doing before we ask it to have a better conversation."

Week 12 Adolescent and ageing brains

Adolescence. Robust: increased reward sensitivity, heightened peer influence, gradual maturation of regulatory systems. Popularly overstated: "the adolescent brain is not fully formed until 25" is a simplification that can infantilise young people.

Ageing. Robust: processing speed and some memory functions decline; crystallised knowledge and emotional regulation often improve; social selectivity increases. Popular: "the brain stops changing" neuroplasticity continues.

Clinical use. Match expectations to capacity without lowering them; scaffold rather than lecture; and design sessions around the person's best time of day.

Week 13 Neuroplasticity and its limits

Robust. Experience changes the brain; learning involves synaptic change; rehabilitation after injury exploits plasticity.

Oversold. "You can rewire your brain in 21 days"; "neuroplasticity means anything is possible"; specific protocols claiming to rebuild neural pathways in defined sessions.

The clinical use. Neuroplasticity is the reason therapy can work and the reason it takes time. Say that; it is both hopeful and honest.

Week 14 Integration

A physiological formulation answers: what is the person's current state, what regulates them, what dysregulates them, and what does the family do that helps or harms. Add it to the systemic formulation, do not replace it.

The communication standard. If you cannot explain it to a 12-year-old and to a sceptical colleague, you do not understand it well enough to use it.

Glossary

Allostatic load cumulative physiological cost of chronic adaptation to stress. **Co-regulation** regulation of one person's arousal by another's presence and behaviour. **Cortisol** the primary glucocorticoid stress hormone. **HPA axis** hypothalamic pituitary adrenal axis, the endocrine stress pathway. **Interoception** perception of internal bodily states. **Neuroception** unconscious evaluation of safety and threat (Porges). **Polyvagal theory** a theory linking autonomic states to social behaviour. **Window of tolerance** the arousal range within which a person can think and connect.

Self-assessment

Multiple choice

1. Above a certain level of physiological arousal, a person: (a) listens better (b) finds listening and empathy physiologically difficult (c) becomes more rational (d) is unaffected

2. Co-regulation means: (a) self-soothing alone (b) one person's calm regulating another's arousal (c) medication (d) breathing only

3. The HPA axis ends with: (a) the amygdala (b) cortisol release from the adrenal glands (c) the hippocampus (d) the vagus nerve

4. Social exclusion: (a) is emotionally neutral (b) activates pain-related circuitry (c) improves resilience (d) has no physiological correlate

5. "The brain is fully formed at 25" is: (a) a precise finding (b) a simplification that can infantilise young people (c) false in every sense (d) irrelevant

6. Polyvagal theory is best described as: (a) fully established (b) clinically useful and partly contested (c) disproven (d) unrelated to therapy

7. Neuroplasticity means that: (a) change is instant (b) experience shapes the brain, over time (c) therapy is unnecessary (d) the brain stops changing at 25

8. A break protocol in couple conflict works because: (a) it avoids the issue (b) it allows arousal to fall before the conversation continues (c) it punishes the partner (d) it is recommended by law

9. Sleep should be assessed because it: (a) is not clinical (b) is robustly linked to mood and regulation (c) only matters for children (d) is a lifestyle preference

10. Physiological explanations should be used to: (a) replace formulations (b) pace sessions, teach regulation and reduce shame (c) assign blame (d) impress families

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Name three robust and one contested claim in the neuroscience of relationships.
2. Describe the HPA axis in five steps.
3. Give a four-step teaching sequence for a regulation skill.
4. Why is "regulate before reason" clinically important in couple conflict?
5. What is the difference between a physiological account and a formulation?

Essay (choose one, 2,000 words)

1. "Neuroscience has been oversold to therapists." Critically evaluate with three examples.
2. Integrate a physiological and a systemic formulation for a couple with escalating conflict.
3. How should a family therapist talk about the brain to a sceptical 15-year-old and a religious grandmother?

Further reading

- Porges, S. W. (2011). The Polyvagal Theory. Norton.

- Siegel, D. J. (2012). The Developing Mind (2nd ed.). Guilford.
- Sapolsky, R. M. (2004). Why Zebras Don't Get Ulcers (3rd ed.). Holt.
- Eisenberger, N. I., & Lieberman, M. D. (2004). Why rejection hurts. Trends in Cognitive Sciences, 8(7), 294 300.
- Cozolino, L. (2017). The Neuroscience of Psychotherapy (3rd ed.). Norton.
- Grossman, P., & Taylor, E. W. (2007). Toward understanding respiratory sinus arrhythmia. Biological Psychology, 74(2), 263 285. (Critique of polyvagal claims.)

Next: MFT 727 Play Therapy Techniques

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 28 OF 28

MFT 727

Play Therapy Techniques

14 teaching weeks 3 practicals 1 figure

ELECTIVES

1.71 units Prerequisite: MFT 711

> Play is the child's language and the therapist's instrument.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0

MFT 727 Play Therapy Techniques

MSc Marriage and Family Therapy Elective 1.71 Units 14 teaching weeks Prerequisite: MFT 711

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 727 Play Therapy Techniques. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	**Play Therapy Techniques**	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The fourteen-week plan . . . 6

Week 1 What play is for . . . 6

Week 2 The therapeutic powers of play . . . 7

Week 3 The room and the toys . . . 7

Week 4 Limit setting . . . 8

Week 5 Non-directive skills I . . . 8

Week 6 Non-directive skills II . . . 9

Week 7 Observed non-directive session . . . 9

Week 8 Directive techniques . . . 9

Week 9 Sandtray . . . 10

Week 10 Assessment through play . . . 10

Week 11 Trauma and play; dissociation . . . 11

Week 12 Filial therapy . . . 11

Week 13 Family play therapy . . . 12

Week 14 Limits, evidence and integration . . . 12

Glossary . . . 12

Self-assessment . . . 13

- Multiple choice . . . 13
- Answer key . . . 13
- Short answer . . . 13
- Essay (choose one, 2,000 words) . . . 13

Further reading . . . 14

Course leader's summary. Play is the child's language and the therapist's instrument. This course trains you to set up a room, hold a session, use directed and non-directed methods, involve parents as partners, and recognise when play is not enough.

How to use this book

Play therapy is learned in a room, not in a book but a badly equipped room produces bad therapy. Work through the weeks in order: room, stance, skills, methods, family application, limits. Practise every technique on a peer before you use it with a child.

1. Course description

The course covers the history and theory of play therapy; the child-centred (non-directive) model and its core skills; directive and cognitive-behavioural play; sandtray; art and puppetry; filial therapy (teaching parents to be therapeutic agents); family play therapy; assessment through play; trauma and play; and the evidence base.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the theory and evidence for play therapy	Exam
LO2	Set up a safe, well-equipped playroom	Room plan and rationale
LO3	Conduct a non-directive session using core skills	Observed session
LO4	Use at least three directive techniques appropriately	Skills portfolio
LO5	Teach parents filial skills	Parent session observed
LO6	Assess through play without over-interpreting	Case study
LO7	Recognise limits and refer	Viva

3. Assessment

Component	Weight	Due
Playroom plan with item rationale (1,200 words)	15%	Week 4
Observed non-directive session (30 min)	30%	Week 7
Techniques portfolio (five techniques, with reflection)	20%	Week 10
Case study including filial work (2,500 words)	25%	Week 13
Examination	10%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	What play is for; history and theory	Free play observation
2	The therapeutic powers of play	Session structure mapping
3	The room and the toys	Room setup
4	**Room plan due**; limit setting	Limit-setting rehearsal
5	Non-directive skills I: tracking and reflecting	Tracking drill
6	Non-directive skills II: returning responsibility, esteem-building	Skills drill
7	**Observed non-directive session**	Assessed
8	Directive techniques: art, puppets, role-play, stories	Technique practice
9	Sandtray	Sandtray practice
10	**Portfolio due**; assessment through play	Assessment exercise
11	Trauma and play; dissociation in the playroom	Pacing and grounding
12	Filial therapy: parents as agents	Parent coaching
13	**Case study due**; family play therapy	Family session design
14	Limits, evidence and integration	Viva

Week 1 What play is for

Theory. Play is children's natural medium for expression, mastery, and meaning-making. Axline's eight principles (1947) remain the most quoted statement of the non-directive stance: the therapist develops a warm relationship; accepts the child as they are; establishes permissiveness; recognises and reflects feelings; respects the child's capacity to solve problems; does not direct; does not hurry; and sets only those limits that are necessary.

Why it works (proposed mechanisms). Play allows symbolic expression of experience that cannot be verbalised; it provides a sense of control and mastery; it allows rehearsal of difficult situations; and the relationship with the therapist provides a corrective relational experience.

Evidence. Play therapy has moderate support for a range of child problems, with stronger effects when parents are involved (filial therapy) and when the approach is matched to the problem (e.g. CBPT for anxiety). It is not a panacea, and it is not a substitute for trauma-specific treatment where indicated.

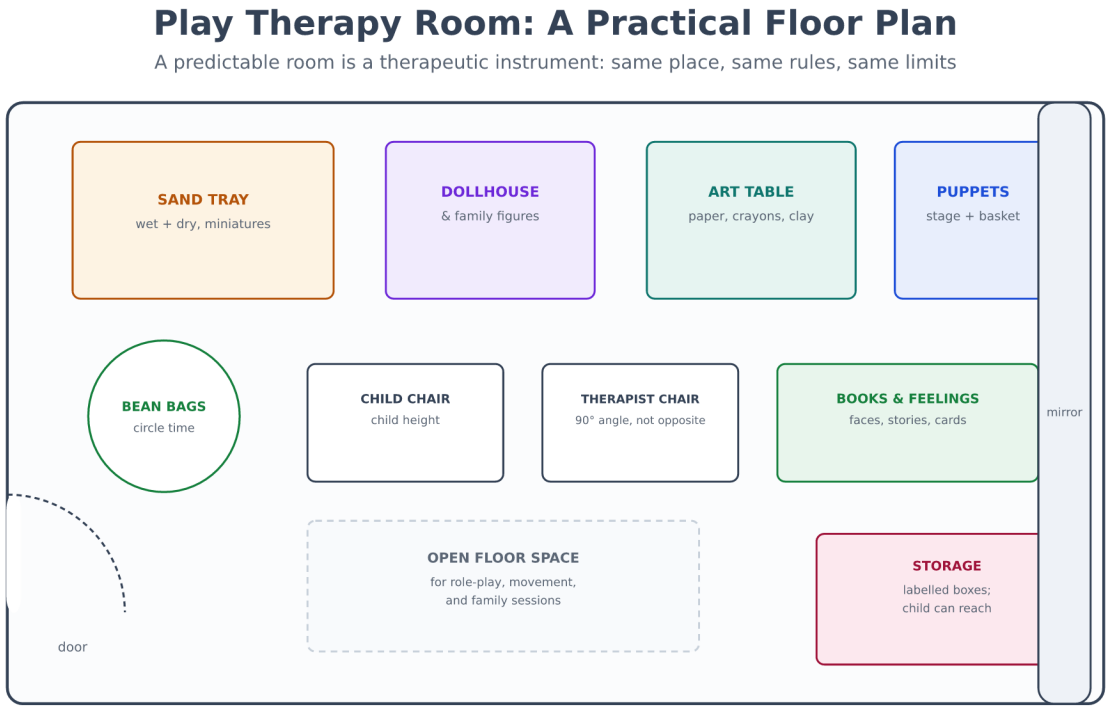
> **PRACTICAL**
> Observe a child in free play for 15 minutes. Record behaviours only: what they chose, what they returned to, what they avoided, how they ended.

Week 2 The therapeutic powers of play

Power	What it looks like	How the therapist supports it
Communication	The child enacts the problem	Track and reflect
Emotional regulation	Repetitive calming play	Allow repetition; contain
Mastery	Replaying a frightening event with a different ending	Do not interpret; note the shift
Symbolic distance	The "monster" rather than the father	Stay in the metaphor
Relationship	The child tests and then trusts	Consistency, warmth, limits
Imagination	New possibilities	Follow, do not lead

Session shape for a child. Opening ritual (same each time), free play, a focused activity, a closing ritual. Predictability is the intervention.

Week 3 The room and the toys



NOTE

Limit-setting script: 'You can be as angry as you want in here, and you may not hurt yourself, me, or the room.' State the limit, offer an acceptable alternative, and name the feeling. Consistency is the intervention.

Figure 3.1 A play therapy room: layout, furniture and sightlines

Principles. Safety, predictability, accessibility, and privacy. The child should be able to reach what they need; the therapist should be able to see everything.

Toy categories (Landreth's classification).

1. **Real-life toys** dolls, family figures, cars, a cash register, a phone, cooking utensils.
2. **Acting-out/aggressive toys** toy soldiers, a punch bag, wild animals, a toy gun, dinosaurs.
3. **Creative/expressive toys** crayons, paint, clay, sand, blocks, musical instruments.

What to include. Sand tray, water, dolls and family figures, animal families, puppets, art materials, blocks, a dollhouse, a medical kit, a small house, blankets, and a soft toy.

What to avoid. Toys that require the therapist's help to operate; toys with one "right" answer; electronic toys that dominate; and anything that could be used to hurt without a limit.

Room rules posted at child height. "We stay in the room. We do not hurt ourselves. We do not hurt each other. We do not hurt the room. We put things away when we finish."

Week 4 Limit setting

Why limits matter. Limits create safety, teach self-control, and protect the relationship. Permissiveness without limits is not acceptance; it is abandonment.

The ACT model (Landreth).

- **A** **Acknowledge the feeling:** "You are really angry with me."
- **C** **Communicate the limit:** "But I am not for hitting."
- **T** **Target an acceptable alternative:** "You can hit the cushion, or you can tell me with words."

Sequence. State the limit calmly, once; repeat with the alternative; if it continues, offer a choice ("you can choose to put the toy down or I will put it away"); then follow through without anger.

What not to do. Negotiate; explain at length; threaten consequences you will not enforce; or shame the child.

> **REHEARSAL**
> Role-play a child who throws a toy at you, three times. The third attempt should be delivered with the same calm as the first.

Week 5 Non-directive skills I

Tracking narrating behaviour without evaluation: "You are putting the blue block on top. It wobbles."

Reflecting content "You want the mother doll to go away."

Reflecting feeling "You look pleased with that." / "That made you angry."

Rules of thumb.

- Describe, do not question.
- Name feelings tentatively ("you seem "), allowing correction.
- Follow, do not lead.
- Stay in the metaphor; do not translate ("so the monster is your father" is an interpretation, not a reflection).

> **DRILL**
> Ten minutes with a peer playing a 6-year-old. You may only track and reflect. No questions. The observer counts interpretations (target: zero).

Week 6 Non-directive skills II

Returning responsibility handing the problem back: "That is something you can decide." / "You can try it."

Esteem-building attributing effort and choice, not talent: "You kept trying until it stood up."

Facilitating self-direction "You know what you want to do next."

Enlarging the meaning offering a slightly wider frame: "You are the one who decides what happens in this story."

The therapist's own regulation. Children notice your body. Breathe slowly, sit at their level, keep your face open, and speak more slowly than you think you should.

Week 7 Observed non-directive session

Thirty minutes with a trained child actor (or a filmed role-play marked against criteria). Assessed on: room and structure; tracking and reflection; returning responsibility; one limit set well; warmth; and the closing ritual.

Week 8 Directive techniques

Directive play is not instruction. It is the deliberate use of a medium to address a specific issue, always following the child's lead within the structure.

Technique	Use	How
Drawing (family, house, fears, "worst day")	Assessment and expression	"Draw your family doing something." Then ask them to tell you about it

Technique	Use	How
Puppets	Safe displacement	Let the child choose a puppet; interview the puppet, not the child
Role-play	Rehearsal and mastery	"Show me what happens at the school gate"
Bibliotherapy	Normalising and skills	Read a story about a similar problem; discuss the character
Feelings cards / thermometer	Emotional vocabulary	Sort into "big/little", "now/then"
CBPT	Anxiety, trauma	Psychoeducation, coping skills, graded exposure in play

Sequence for a directive activity. Explain briefly demonstrate let the child lead observe reflect close.

Week 9 Sandtray

What it is. A tray of dry and wet sand with miniature figures, used to create a world.

Two traditions. Non-directive (the child builds, the therapist witnesses and reflects) and directive (therapist suggests a theme, e.g. "make a world where you feel safe").

Practicalities. Trays with a water-resistant base; dry sand and water; a wide range of miniatures (people, animals, buildings, vehicles, natural objects, fantasy, symbols of spirituality); a camera to record the scene (with consent); and a clear policy about dismantling.

Interpretation discipline. Do not impose symbolic meanings. Ask: "Tell me about this world." "What happens next?" "Who is this?" The child is the authority on their world.

With older children and adults, sandtray can be used as a non-verbal way of representing a family system who is placed near whom, what is buried, what is outside the tray.

Week 10 Assessment through play

What you can learn. Developmental level (language, symbolic play, attention, motor control); emotional expression; themes (abandonment, violence, rescue, repetition); relational capacity (can they play with you?); and the impact of specific events.

What you cannot learn reliably. Specific causation; the identity of an abuser; or a diagnosis from a single drawing. Any assessment claim must be cautious and triangulated with other data.

Signs warranting attention. Play that is repetitive, narrowed and joyless; sexualised play beyond developmental norms; play involving harm to self or others; a sudden change in play after an event; and a child who cannot play at all.

Documentation. Describe behaviour and the child's own words; avoid interpretation in the record.

Week 11 Trauma and play; dissociation

Principles (from MFT 714). Safety and stabilisation first; the caregiver's regulation is the child's best medicine; do not force the narrative; and expect repetition.

In the playroom. Watch for signs of dissociation (stillness, glazed eyes, going blank, sudden compliance). Slow down, orient to the room, offer a grounding object, and return to safe play.

Post-traumatic play is repetitive and lacks resolution. The therapeutic task is not to stop it but to help it move by reflecting feeling, introducing a small element of mastery, and involving the caregiver.

When play is not enough. Severe trauma symptoms, self-harm, dissociation, and complex abuse histories need trauma-specific treatment (e.g. TF-CBT) alongside or after stabilisation.

Week 12 Filial therapy

What it is. Training parents to conduct child-centred play sessions at home, with supervision. Developed by Guerney; extensively researched; strong evidence for improving parent child relationships and reducing child problems.

Why it works. It increases parental empathy and reflective function, changes the parent's view of the child, and provides a structured positive interaction that most distressed families lack.

The structure (typically 10 sessions).

1. Teach the principles and skills (tracking, reflecting, limit setting).
2. Demonstrate a play session with the child while the parent observes.
3. The parent conducts a session while the therapist coaches.
4. The parent conducts sessions at home (30 minutes weekly) and reports back.
5. Review and generalise.

Practical adaptations. Limited space at home; other children; a parent who works nights; a parent with their own trauma history (support first).

> **PARENT SESSION**
> Coach a parent through a 15-minute play session using tracking, reflection and one limit.

Week 13 Family play therapy

Why involve the family. The child's problem exists in a system; play can make the system visible.

Techniques.

- **Family drawing and sculpture** who sits where, who is close, who is left out.
- **Co-constructed play** the family builds one world together; observe negotiation, hierarchy and exclusion.
- **Role reversal** the child directs the parents in a scene; useful for revealing the child's experience.
- **Structured family games** cooperative tasks that require negotiation.

Caution. Family play can expose conflict. Have a structure for containing it, and be ready to end the activity and debrief.

Week 14 Limits, evidence and integration

The limits of play therapy. Not a substitute for medical assessment, safeguarding action, trauma-specific treatment or family work where the family system is the problem. Play therapy with a child in a family that continues to harm them is not treatment; it is containment.

The evidence summary. Moderate support across problems; stronger when parents are involved; strongest when the approach is matched to the problem and delivered with fidelity.

Integration. Play therapy sits comfortably within a systemic frame when it is used as the child's channel of communication within a family assessment, and when parents are brought into the work.

Glossary

ACT model Acknowledge, Communicate the limit, Target an alternative. **Child-centred play therapy** non-directive approach following the child's lead (Axline, Landreth). **CBPT** cognitive-behavioural play therapy. **Filial therapy** training parents to conduct child-centred play sessions. **Post-traumatic play** repetitive, unresolving play following trauma. **Sandtray** a tray of sand with miniatures used for symbolic expression. **Therapeutic powers of play** the mechanisms through which play produces change.

Self-assessment

Multiple choice

1. The ACT model stands for: (a) Acknowledge, Communicate, Target (b) Ask, Calm, Tell (c) Assess, Contain, Treat (d) Attend, Connect, Teach

2. In non-directive play therapy the therapist: (a) leads the play (b) follows the child's lead (c) interprets symbols (d) teaches skills

3. Telling a child "the monster is your father" is: (a) a reflection (b) an interpretation and generally to be avoided (c) tracking (d) limit setting

4. Filial therapy involves: (a) the child alone (b) training parents to conduct child-centred sessions (c) medication (d) group therapy

5. Post-traumatic play is typically: (a) joyful (b) repetitive and without resolution (c) absent (d) directed by the therapist

6. Sandtray interpretation should be: (a) imposed by the therapist (b) led by the child's own account (c) avoided entirely (d) symbolic decoding

7. Limits in play therapy exist to: (a) control the child (b) create safety and teach self-control (c) save toys (d) satisfy parents

8. A child's drawing alone: (a) diagnoses abuse (b) cannot diagnose or identify an abuser (c) is irrelevant (d) replaces assessment

9. Play therapy is contraindicated as a substitute for: (a) warmth (b) safeguarding action and trauma-specific treatment (c) structure (d) toys

10. The strongest play therapy effects are found when: (a) the child is seen alone for a year (b) parents are involved and the approach matches the problem (c) the therapist interprets (d) sessions are longer

Answer key

1 a 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. State Axline's principles in your own words (five of the eight).
2. List the three categories of toys with two examples of each.
3. Give the ACT sequence with a scripted example.
4. What are the five steps of a child-centred session structure?
5. When is play therapy not enough, and what do you do?

Essay (choose one, 2,000 words)

1. "Play is the child's language." Discuss the implications for assessment and treatment.
2. Design a play therapy service for children in a Nigerian primary school, including the room, the toys, the referral pathway and the evidence.

3. Analyse a play session (yours or a recorded one) using the core skills, and identify what you would do differently.

Further reading

- Axline, V. M. (1947/1969). Play Therapy. Ballantine.
- Landreth, G. L. (2012). Play Therapy: The Art of the Relationship (3rd ed.). Routledge.
- VanFleet, R., Sywulak, A. E., & Sniscak, C. C. (2010). Child-Centered Play Therapy. Guilford.
- Bratton, S. C., Ray, D., Rhine, T., & Jones, L. (2005). The efficacy of play therapy with children: A meta-analytic review. Professional Psychology: Research and Practice, 36(4), 376 390.
- Gil, E. (2017). Play in Family Therapy (2nd ed.). Guilford.

End of the elective sequence. Return to the Programme Handbook.

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 22 OF 28

MFT 770

Master Thesis

14 teaching weeks · 2 practicals

SEMESTER THREE

1.71 units · Prerequisite: MFT 705, 721

> “The thesis is not a bigger essay.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

CC BY 4.0 · 2026 edition

MFT 770 — Master Thesis

MSc Marriage and Family Therapy · Semester Three (continuing) · 1.71 Units · 14 structured weeks + submission and defence · Prerequisite: MFT 705, 721

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

Lecture Library contributors (2026). MFT 770 — Master Thesis. MSc Marriage and Family Therapy lecture library. CC BY 4.0.

Licence

Creative Commons Attribution 4.0 International (CC BY 4.0). Share and adapt freely, including commercially, with attribution. Every figure in this volume was generated by the scripts in `scripts/`; there are no third-party images.

A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	**Master Thesis**	Three	1.71

Contents

How to use this book . . . 5

1. What the thesis is . . . 5

2. The fourteen structured weeks . . . 5

3. Choosing a question you can finish . . . 6

4. The literature review . . . 6

5. Design and methods . . . 7

6. Ethics . . . 7

7. Data collection . . . 8

8. Analysis . . . 8

9. Writing the thesis . . . 8

10. The supervision relationship . . . 9

11. The viva . . . 9

12. After the defence . . . 10

13. Project management . . . 10

14. Assessment . . . 10

Appendix: thesis checklist . . . 11

Further reading . . . 11

Course leader's summary. The thesis is not a bigger essay. It is a supervised, ethical, original inquiry that a professional body would accept as evidence that you can generate knowledge, not only consume it. This book is the project management manual plus the academic guide.

How to use this book

Work the milestones in order; they are dated to protect you from the two classic disasters — a brilliant plan that cannot be completed, and a completed study that answers nothing. Bring the weekly artefact to supervision even if it is imperfect. Progress beats polish.

1. What the thesis is

Element	Requirement
Length	15,000–20,000 words (excluding appendices), or the institutional equivalent
Contribution	Original, however modest: new data, new analysis, a local validation, a qualitative account, a service evaluation
Methods	Quantitative, qualitative or mixed; must be defensible and feasible
Ethics	Institutional approval before any data collection
Supervision	Minimum fortnightly contact with a named supervisor
Output	Bound thesis, oral defence, and (encouraged) a publishable manuscript

What it is not. A literature review alone (that is a different degree component). A clinical case series without a systematic method. A replication of a manual with no question. A project that requires resources you do not have.

2. The fourteen structured weeks

Week	Milestone	Artefact due in supervision
1	Topic refinement and scope	One-page problem statement
2	Literature search strategy	Search log and 20 key references
3	Critical literature review draft	2,000-word draft
4	Research question and aims	PICO/SPIDER and aims table
5	Design selection	Design decision log
6	Measurement and materials	Instrument pack
7	Ethics application	Completed ethics form
8	Sampling and recruitment plan	Recruitment protocol and letters
9	Analysis plan	Pre-specified analysis plan

Week	Milestone	Artefact due in supervision
10	Pilot or feasibility work	Pilot findings and amendments
11	Data collection checkpoint	Recruitment and data quality report
12	Analysis progress	First results tables/coding frame
13	Findings and discussion drafting	Draft chapters 4–5
14	Full draft and defence preparation	Complete draft, viva questions

Timeline discipline. A part-time student should assume 12–18 months from registration to defence. Full-time students commonly need 9–12 months. If your plan assumes everything goes right, add 30%.

---

3. Choosing a question you can finish

The three-circle test. A viable thesis sits where (1) you are curious, (2) the field needs it, and (3) you have access. If any circle is missing, choose a different question, not a bigger effort.

Good thesis questions in this field (examples).

1. What is the reliability and factor structure of the RDAS in a Nigerian clinical sample?
2. How do fathers describe their experience of being excluded from family therapy?
3. Does a four-session psychoeducation group reduce caregiver burden among families of people with psychosis?
4. What do couples report about the process of rebuilding trust after infidelity?
5. What predicts drop-out after a first family therapy appointment in a community clinic?
6. How do therapists in training describe the influence of their own family of origin on their clinical decisions?
7. Does a culturally adapted version of the speaker–listener technique improve communication in a small clinical series?

Questions to avoid. "The effect of family therapy on family functioning" (too broad); "Why is family therapy better than individual therapy?" (advocacy, not inquiry); anything requiring a sample you cannot access or an instrument you cannot obtain.

> **ARTEFACT**
> A one-page problem statement: what is unknown, why it matters, who it affects, and what you will produce.

---

4. The literature review

Purpose. Not to summarise everything, but to establish (a) what is known, (b) what is contested, (c) what is missing, and (d) why your question follows.

Search discipline.

- At least three databases (PubMed, PsycINFO, Scopus, AJOL, Google Scholar as a supplement).
- Document the strategy: terms, limits, date, number of results, number retained.
- Use a reference manager from day one.
- Read systematic reviews first; they map the field faster than primary papers.

Structure of the chapter. Context → key constructs and definitions → what the evidence shows → what is contested → what is missing (the gap) → your question and aims.

The gap must be specific. "Few studies have examined X in Y population using Z method" — not "more research is needed."

---

5. Design and methods

The rule. The method chapter must be detailed enough that a competent reader could replicate the study, and honest enough that they could see its limits.

Chapter contents.

1. Design and rationale.
2. Setting and context.
3. Participants: inclusion, exclusion, recruitment, consent, sample size justification.
4. Measures: source, psychometrics, scoring, adaptation and translation procedures.
5. Procedure: what happened, in what order, for how long.
6. Analysis: software, models, assumptions, missing data, coding approach, rigour procedures.
7. Ethical considerations: approval reference, consent, confidentiality, risk, data storage.
8. Researcher position and reflexivity.

---

6. Ethics

Required before any data collection. A retrospective application is a serious breach.

Standard components. Purpose and design; participants and recruitment; consent process and documents; risks and benefits; confidentiality and data management; payment; dissemination; and procedures for distress or incidental findings.

Family-specific issues. Consent from guardians plus assent from minors; separate consent for each family member; managing the possibility that one member withdraws; protecting group confidentiality; and a protocol for disclosures of risk during data collection.

ARTEFACT

A complete ethics application with consent forms for an adult and a minor, and a risk protocol for a participant who discloses harm during an interview.

7. Data collection

Pilot first. Five to ten participants will reveal problems that a hundred will not forgive: unclear questions, unworkable recruitment, unusable audio, unrealistic time estimates.

Data quality. Check recordings the same week; transcribe promptly; keep a fieldwork log; record refusals and reasons (they are data).

Recruitment tactics that work in clinical settings. A named contact who can introduce you; a one-page information sheet; flexible times; a clear statement of what participation involves; and a follow-up call at 48 hours. Do not rely on posters.

8. Analysis

Quantitative. Follow the pre-specified plan; document deviations; check assumptions; report descriptives before inference; report effect sizes with confidence intervals; run sensitivity analyses; keep the analysis log current.

Qualitative. Transcribe verbatim; code systematically; keep memos; audit your coding; discuss divergence with a second coder; move from codes to themes with an explicit, documented logic; and present themes with data extracts (anonymised).

Mixed methods. Decide in advance how the strands will be integrated, and produce at least one joint display.

9. Writing the thesis

Standard structure.

Chapter	Content	Typical length
1	Introduction and background	2,000
2	Literature review	5,000
3	Methodology	3,500
4	Findings	4,000
5	Discussion	3,500
6	Conclusion and recommendations	1,000
—	References, appendices	—

Writing rules.

- Write the method first; it is the easiest chapter and it clarifies everything.
- Draft findings as soon as you have data; do not wait for "enough".
- Keep a "parking lot" file for ideas that belong elsewhere.
- Write 500 words a day rather than 5,000 in a panic.
- Read your work aloud; if you run out of breath, the sentence is too long.

Voice. Academic but human. First person is acceptable in reflective and qualitative sections; follow your institution's guidance. Avoid both inflated jargon and chatty informality.

---

10. The supervision relationship

What your supervisor owes you. Regular contact; timely feedback; methodological and ethical guidance; honesty about the standard of your work; and reasonable availability.

What you owe your supervisor. Prepared meetings with an agenda and an artefact; drafts submitted on time; decisions recorded; and honesty about difficulties — especially delays.

Managing difficulty. If supervision is not working, raise it early with the programme director. A change of supervisor is legitimate and common; drifting for six months is not.

The meeting agenda template.

1. Progress since last meeting (three bullets).
2. Decisions needed today (the most important item first).
3. Problems I am stuck on.
4. What I will do before the next meeting.
5. Date of next meeting.

---

11. The viva

What examiners test. Whether you understand your own study: why you chose it, what you did, what it means, and what it cannot support.

Preparation.

1. Re-read your thesis and mark every claim you could not defend.
2. Prepare a two-minute summary.
3. Prepare answers to the four standard questions: design, validity, analysis, conclusion.
4. Rehearse with a peer who is instructed to be difficult.
5. Bring a copy of your thesis, your analysis log and a pen.

In the room. Answer the question asked. Say "I don't know" when you don't. Distinguish what the data show from what you hope they show. Correct errors gracefully.

Common reasons for referral or failure. Design cannot answer the question; analysis misapplied; ethics breach; unacknowledged limitations; and evidence of work not being the candidate's own.

12. After the defence

Corrections. Make them promptly, with a tracking table mapping each examiner comment to the change made.

Dissemination. Submit a manuscript to a journal; present at a conference; give a plain-language summary to the participating service. Dissemination is part of the ethical contract with your participants.

Your data. Retain as required by your institution's policy, then destroy securely.

13. Project management

Risk	Early warning	Mitigation
Recruitment failure	Fewer than 50% of target by mid-point	Widen eligibility; add sites; adjust design (with supervisor and ethics approval)
Analysis paralysis	Repeated re-coding without writing	Fixed decision date; write the methods and limitations
Supervisor unavailability	Missed meetings	Written agenda; escalate early
Personal crisis	Missed deadlines	Tell the programme early; request extension with evidence
Scope creep	New questions each week	Freeze the question after Week 4; park ideas

The one-page dashboard. Question, milestones, current position, next action, date, supervisor appointment. Update weekly; it will save your degree.

14. Assessment

Component	Weight
Proposal (approved by panel)	10%
Ethics approval	Pass/fail gate
Thesis	70%
Oral defence	20%

Marking criteria (typical). Originality and significance; command of the literature; appropriateness and rigour of method; quality of analysis; critical discussion; ethical practice; presentation and referencing.

Appendix: thesis checklist

- ■ Question is specific, answerable, and stated in one sentence
- ■ Literature search documented and reproducible
- ■ Gap statement is specific, not "more research is needed"
- ■ Design justified against at least one alternative
- ■ Sample size justification provided
- ■ Ethics reference number included
- ■ Measures described with psychometric properties and any adaptation
- ■ Analysis plan pre-specified; deviations recorded and explained
- ■ Assumptions checked and reported
- ■ Effect sizes with confidence intervals
- ■ Findings presented before interpretation
- ■ Limitations specific and honest
- ■ Implications for practice are actionable
- ■ Referencing consistent and complete
- ■ Plain-language summary prepared for participants

Further reading

- Wisker, G. (2012). The Good Supervisor. Palgrave.
- Booth, W., Colomb, G., & Williams, J. (2016). The Craft of Research (4th ed.). Chicago.
- Swetnam, D., & Swetnam, R. (2009). Writing Your Dissertation. How To Books.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Field, A. (2018). Discovering Statistics Using IBM SPSS Statistics (5th ed.). Sage.

End of Semester Three. Continue to the electives: MFT 722 — Professional Development and Self-Care